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Discrepancies between registration record and publication in COVID-19 related RCTs carried out in China mainland

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Title: Discrepancies between registration record and publication in COVID-19 related RCTs carried out in China mainland

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Summary boxes

Section 1: What is already known on this topic

Transparent reporting in RCTs is important as RCT-based evidence is most valued in decision making. In 2009, a study showed that among properly registered RCTs in high impact factor journals, 31% trial reports had different levels of discrepancies between registered and reported primary outcomes. To present day, the consistency between registered and published trial information regarding COVID-19 related RCTs conducted in mainland China has not been evaluated.

Section 2: What this study adds

87.5% published protocols and 97.4% reports had different levels of discrepancies (or missing information) between registration and publication. Discrepancies of primary outcome was present in 62.5% published protocols and 69.2% reports. These results highlighted the need to improve reporting transparency in RCTs.

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Abstract

Objective

To evaluate the reporting transparency of COVID-19 related RCTs carried out in mainland China through comparison between registration records and publications.

Design

Complete lists of COVID-19 related trials were obtained from International Clinical Trials Registry Platform (ICTRP) (updated on 13 May 2021). Chinese Clinical Trial Registry (ChiCTR), ClinicalTrials.gov (NCT), the ISRCTN registry (ISRCTN), and EU Clinical Trial Register (EUCTR) were searched for additional COVID-19 related trials on May 16th 2021. Only RCTs carried out in mainland China were included. Corresponding publications were searched with trial registration number in PubMed, Embase, Cochrane Library, CNKI.net and Wanfangdata. Data were extracted from registration records and publications. Risk of bias assessment (RoB2) was performed for trial reports and the information provided by registration and publications was compared for consistency.

Results

360 registrations were included, in which 16 studies were repeatedly registered, making the number of eligible studies 344. From these studies 16 protocols and 39 RCT reports were published. In RoB2 assessment, overall, high risk was found in 18 (46.2%) studies; 17 (43,6%) studies had some concerns; 4 (10.3%) were at low risk. 14 (87.5%) protocols and 38 (97.4%) reports had different levels of discrepancies (or missing information) between registration and publication. Discrepancies distributed

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in multiple domains including sample size (6, 37.5% for protocols and 23 60.0% for reports, same below), trial setting (9, 56.3% and 56.4%), inclusion criteria (10, 62.5% and 30, 76.9%), exclusion criteria (9, 56.3% and 28, 71,8%), masking method (7, 43.8% and 20, 51.3%) and primary outcome or time frame of primary outcome measurement (10, 62.5% and 27, 69.2%).

Conclusions

The high rate of discrepancy between registration records and trial reports revealed compromised transparency in reporting of COVID-19 relevant RCTs performed in China mainland. The importance of trial registration should be further emphasized to enhance transparent RCT reporting.

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Strengths and limitations of this study

- The study provided a full coverage of publicly registered RCTs related to the prevention, treatment or prognosis of COVID-19 infection.
- The study identified publications citing these registration records and examined the consistency of methodology between registration and publication.
- The study included only RCTs performed in mainland China.
- RCT reports that did not cite any registration number (if any) were not included in the analysis.

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Introduction

Presumably, most readers of this article are familiar with the concept of Evidence Based Medicine[1]. On the top hierarchy of evidence stand Randomized Controlled Trials (and the meta-analyses based on them) and their importance cannot be overemphasized. However, not all RCTs are of the same quality [2], which leads to one important question: the transparency in RCT reporting.

"The whole of medicine depends on the transparent reporting of clinical trials."[3] Transparent reporting requires a complete description of methodology through which the trial data are collected and analyzed, a report without omitting any data generated by the trial and a standard way of writing [2]. Much has been done to address this problem, including ICMJE member's requirement for registration in public trial registry prior to enrollment [4], the announcement of CONSORT statement [5], and the mandatory submission of trial protocol enforced by journals with high impact factors. However, the overall report transparency of RCT remains suboptimal.

In evaluation of transparency, trial registry plays a special role, since other documents submitted to editors are, in methodologists' point of view, subject to modification, whereas registration information, with well documented history versions and time stamp, may reflect trialists' original intention more accurately. In this study, we aimed to perform analysis on the transparency of COVID-19 related RCTs performed in China mainland based on a comprehensive consideration of trial registry information, published trial

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protocols and reports.

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Methods

International Clinical Trials Registry Platform (ICTRP), Chinese Clinical Trial Registry (ChiCTR), ClinicalTrials.gov (NCT), the ISRCTN registry (ISRCTN) and EU Clinical Trial Register (EUCTR) were accessed on May 16th 2021. A complete list of COVID-19 trials in csv format, which was updated on May 13th 2021, was retrieved from ICTRP. Filters were applied to restrict study types to interventional studies and studies related to prevention, treatment or prognosis of COVID-19 infection. Studies were excluded if they were of observational design (case studies, case-control studies, cohort studies, cross-sectional studies), non-randomized (including quasi-randomized), without a control group or carried out outside China mainland. For multicenter trials, all centers must be located in China mainland to be eligible for analysis. From ChiCTR, Index of studies of Novel Coronavirus Pneumonia (COVID-19) updated on May 12th 2021 was obtained. From NCT, listed clinical studies related to the coronavirus disease (COVID-19) were accessed and study type was restricted to “Interventional (Clinical Trial)”, and map panel was used to select studies performed in China mainland. For ISRCTN and EUCTR, searches were conducted manually with keywords “COVID-19”, “covid19”, “SARS-Cov-2” or “2019-nCov” and the results were screened for eligibility according to above mentioned criteria. Next, eligible registration records were pooled for analysis.

Search for publication which cited these registration records was performed on the same day at PubMed, Embase and Cochrane Library, and two frequently used databases for Chinese language literature, CNKI.net and Wanfangdata, using registration ID as unique

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identifiers. From registration records, registration ID, date of registration, date of last update, date of first enrollment, scientific/official title, objective, intervention of interest, comparator, primary purpose, recruitment status, estimated enrollment, arms, registration status (ChiCTR only), ethical approval information (ChiCTR only), name and location of centers, randomization (ChiCTR only), masking, inclusion and exclusion criteria, primary and other outcomes and other relevant information were extracted. From published protocols and trial reports, date of first enrollment, estimated enrollment, arms, center names, inclusion/exclusion criteria, primary/secondary outcomes and information regarding randomization and masking were extracted. In addition, risk of bias assessment for published trial records was performed with RoB 2 [6]. Screening, data extraction and risk of bias assessment were independently performed by two reviewers and consensus was reached through discussion.

For repeated registration records of same trials, information extracted from different registries was compared to determine if discrepancies were present. Repeated registration was identified through similarity of title and identity of primary sponsor. For multi-registered trials, only the record cited by publication and (if all/none of repeated registration records was cited) the record with most recent update time shall enter further analysis process.

Characteristics of eligible registrations, including trial registry, registration status, primary purpose, recruitment status, if the trial was multicentered or suspended, eligible

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participants, number of arms, estimated enrollment, number of primary/secondary outcomes, masking status and publication status were summarized and presented as Number (%) or Median (Interquartile Range) depending on categorical or quantitative nature of the data. Scatter plot and line chart were plotted to illustrated the trend of newly registered trials in China mainland and different provinces. Finally, from published trial protocols and reports, information regarding estimated enrollment, center name, inclusion/exclusion criteria, masking, randomization, primary/secondary outcome and date of first enrollment (report only) was extracted and compared with registration records, among which the consistency was evaluated and summarized.

Patient and Public Involvement

Neither patients nor the public was involved in the design, or conduct, or reporting, or dissemination plans, since the nature of this study made these involvements inappropriate.

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Results

A total of 360 registration records met eligibility criteria, in which 16 studies were repeatedly registered on ChiCTR and NCT, making the number of eligible studies 344. The flow chart of screening process is shown in ***Figure 1***. Among all included studies, 270 (78.5%) were registered on ChiCTR, 73 (21.2%) on NCT and 1 (0.3%) on ISRCTN. 205 (59.6%) of them were prospectively registered (defined as registration date prior to or on date of first enrollment) and 139 (40.4%) were retrospectively registered. The primary purposes for most studies were prevention (60, 17.4%) or treatment (280, 78.5%). By the time of data extraction, 38 (11.1%) studies have completed enrollment process (recruitment status was completed, no longer recruiting or active, not recruiting), 171 (49.7%) were still recruiting (recruitment status was recruiting or enrolling by invitation) and 116 (33.7%) were not yet recruiting. 19 studies were suspended, terminated or withdrawn. 118 (34.3%) were multicenter studies (defined as declaration of more than 1 center in registration record), 218 (63.4%) were single center studies and 8 (2.3%) didn't provide sufficient information. Notably, 19 (5.5%) registrations declared to be multicentered in title or other part of registration but had only one or no specified center. Median estimated enrollment was 120, with maximum of 29,000 and minimum of 12. 248 (72.1%) were two-armed studies, 46 (13.4%) were three-armed, 22 (6.4%) were four-armed and 28 had five or more than five arms. As for eligible participants, 276 (80.2%) recruited clinical diagnosed COIVD-19 infected patients, 7 (2.0%) recruited suspected cases or close contacts of diagnosed patients, while 61 (17.7%) recruited participants not infected by COVID-19. As for masking, 139 (40.4%) studies didn't specify

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any masking method, 105 (30.5%) were open-label studies and (any kind of) masking was adopted in 100 (29.1%) studies. Regarding outcomes, 160 (46.5%) trials specified 1 primary outcome in registration record and 181 (52.6%) declared more than one primary outcome. 3 (0.9%) trials had missing primary outcome. Secondary outcome (if any) was specified in 260 (75.6%) studies. The characteristics of included studies based on registration record are summarized in ***Table 1***. To illustrate the trend of increasing trial registration in China mainland and each province within, a line and scatter point chart was plotted. As it shows, the first COVID-19 trial was registered on Jan-23 2020. As the pandemic began in Wuhan, COVID-19 related RCTs concentrated in Hubei province at first and then expanded to other provinces. The coming four months witnessed an explosive increase in number of newly registered trials. This trend reached plateau around June 2020 and was thereafter steadily increasing till the day of data extraction (***Figure 2A***).

A total of 44 reports[7-50] and 16 protocols[51-66] was published from these registered trials, in which 2 studies published both protocol[54,61] and report[32,41]. Among the reports, 5 were excluded from analysis since they were case series studies or retrospective studies[46-50]. Risk of bias assessment was performed in the remaining 39 reports (***Figure 2B)***. Overall, high risk was found in 18 (46.2%) studies; 17 (43,6%) studies had some concerns; 4 (10.3%) were at low risk.

Next, the consistency between published protocol/report and trial registration information

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was examined. In 16 published protocols, 6 (37.5%) had deviation in sample size and 5 (31.3%) had more than 20% deviation; 3 (18.8%) had not specified any center in registration or protocol and 6 (37.5%) had discrepancy in center; More than half of them had deviation in inclusion criteria (10, 62.5%) or exclusion criteria (9, 56.3%); 7 (43.8%) had not specified masking in text or in registration; In terms of primary outcome, 8 (50%) had discrepancies, including newly introduced primary outcome in protocol (5, 31.3%), omitted registered primary outcome (7, 43.8%) and registered primary outcome described as secondary outcome in text (6, 37.5%); only 2 (12.5%) kept full consistency in all of the domains (***Table 2)***.

Among 39 RCT reports eligible for analysis, 23 (60%) had discrepancies in sample size and in 19 (48.7%) of them, deviation was more than 20%; in one study the deviation was because of reporting of interim analysis[36]. Setting was not specified in registry and/or text in 12 (30.8%) reports, and discrepancy was present in 10 (25.6%) reports. 4 (10.3%) and 6 (15.4%) studies had unspecified inclusion criteria or exclusion criteria, respectively, in registry and/or text. In those where eligibility criteria were specified, 26 (66.7%) had deviation in inclusion criteria and 22 (56.4%) had different exclusion criteria compared to registration. 15 (38.5%) had unspecified masking method in registry/text and 5 (12.8%) had deviation in masking method, among which masking was upgraded (open-label changed to single/double-blind, or single-blind changed to double-blind) in 4 (10.3%) studies and downgraded in 1 (2.6%) study. In respect of primary outcome, 2 (5.1%) reports had no specified primary outcome in text or corresponding registration; 19

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(48.7%) reports had deviation in nature of primary outcome, including introduction of new primary outcome in text (8, 20.5%), omitting of registered primary outcome (14, 35.9%), registered secondary outcome claimed as primary outcome in text (4, 10.3%) and registered primary outcome claimed as secondary outcome intext (10, 25.6%). In studies whose nature of primary outcome was consistent, 2 (5.1%) did not specify time frame of primary outcome measurement in registry/text and 4 (10.3%) had deviation in time frame of primary outcome measurement (***Table 2)***. Only 1 (2.6%) study had full consistency in above-mentioned domains between registration and report.

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Discussion

To summarize, 16 protocols and 39 RCT reports published from 344 COVID-19 related RCT registrations carried out in mainland China. Among these publications, 87.5% protocols and 97.4% reports had discrepancies comparing to registration record, in which 62.6% protocols and 69.2% had unspecified/modified primary outcome or time frame of primary outcome measurement. These numbers revealed compromised transparency in RCT reporting.

Given the pivotal role of RCT in evidence-based medicine, transparent reporting has long been a major concern. In the era when mandatory clinical trial registration was not enforced by ICMJE, which was before 2005, the work by Chen et al. reported 62% published trial report had modified, introduced or omitted in comparing the trial protocols approved by the Scientific-Ethical Committees for Copenhagen and Frederiksberg[67]; when similar approach was applied to trials approved for funding by the Canadian Institutes of Health Research, it was found that primary outcome differed between reports and protocols in 40% of the trials [68]. Such prevalence of selective reporting was apparently unacceptable. To ensure transparency in trial reporting and reduce selective reporting, the ICMJE initiative encouraged researchers to make trial information available to the public[4]. Since then, the number of registrations has increased greatly[69,70]. Registration records are useful information to journal editors, reviewers and researchers because they may reflect the researcher's intention at planning stage of the trials, and each modification is accompanied with a time stamp by trustworthy third-party. In most

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cases this is the only way to obtain validated trial protocol change history for reviewers. Certain authors have taken this advantage in their works: Mathieu et al. compared registrations and publications of randomized controlled trials in journals with highest impact factors in cardiology, rheumatology and gastroenterology, finding that 31% or properly registered RCTs had discrepancies between registered and published primary outcomes[71]; according to Rayhill et al., only about a quarter of RCTs published in the core headache medicine journals displayed proper compliance with trial registration[72]. Similarly, our results revealed lack of compliance in primary outcome of 62.6% protocols and 69.2% reports in COVID-19 related RCTs. These results highlighted the need to apply more careful examination to potentially underlying outcome reporting bias, especially for peer-reviewers and journal editors. Meanwhile, it is also important to emphasize all required information by trial registry be properly filled, since missing or unspecified information will not enable further evaluation, thus decreasing overall transparency of reporting. Since the pandemic of COVID is a major public health issue, first-hand evidence is urgently needed to support clinical decision making in the prevention, treatment or prognosis of this disease. However, the compromised transparency of COVID-19 related RCT reporting in Mainland China cast shadow on the value of these evidence.

The possible limitations of this study included: First, the complete lists of COVID-19 related clinical trials were retrieved from ICTRP and ChiCTR and filtered by the labels provided by these lists, thus, the completeness and precision of record screening relied

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on the correctness of these lists; Second, the search of publications for eligible registration record is conducted with trial registration number as unique identifiers, if a publication did not cite any registration number, the article could not be included in analysis. However, considering the requirement for RCT registration is common in journals, we considered these publications to be of a small number (if existed at all). Third, the reviewers only analyzed the most recent updated version of registration record, without considering the historical changes, yet the validity of conclusion of this review is not jeopardized, since this approach tended to overestimate, but not underestimate, the overall transparency.

To emphasize the transparency of RCT reporting and the validity of evidence, cooperation of different stakeholders is required. Principal investigators have the responsibility to ensure that the trial has been registered before first participant is enrolled and that registration record is updated timely whenever the trial protocol is modified; Journal editors and peer reviewers need to carefully check submitted manuscripts and corresponding registration information, demanding explanation whenever discrepancy occurs. If the significance of trial registration can be properly emphasized, we are one more step closer to the goal of full transparent reporting of RCTs.

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Conclusion

The high deviation rate between registration and publications of COVID-19 related RCT conducted in China mainland revealed compromised transparency in trial reporting. The importance for investigators to maintain a timely and detailed registration and for reviewers to routinely scrutinize registration information need to be emphasized.

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Contributorship Statement

Both reviewers (Y.C. and R.Y.) contributed equally in conceptualization, data extraction, analysis, visualization and drafting the manuscript. Both reviewers have read and approved this manuscript.

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Data Sharing Statement

No additional data available.

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Transparency Statement

The authors affirm that the manuscript is an honest, accurate and transparent of the study reported and no important aspect of the study has been omitted.

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Ethics approval from institutional review board was exempted since the study did not involve human participants or animal subjects.

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Figure Legend

Figure 1. *The flow chart shows the screening process for eligible registration records.*

Figure 2. (A) *The trend of registration of COVID-19 related RCT trials in total and each province. When a trial is multicentered and recruited participants in multiple provinces, the trial is considered to take place in these provinces simultaneously. The publication status of these registered trials is also shown.* ***(B)*** *Results of risk of bias assessment (RoB2) are shown for each RCT report included in this review, the risks in each domain and overall risk are also summarized.*

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Table 1. *Characteristics of all included studies.*

Characteristic	Value	Proportion (%)
Count of Eligible Studies	344	100
Registry		
ChiCTR	270	78.5
Clinicaltrials.gov	73	21.2
ISRCTN	1	0.3
Registration Status		
Prospective	205	59.6
Retrospective	139	40.4
Primary Purpose		
Prevention	60	17.4
Treatment	270	78.5
Prognosis	3	0.9
Quality of Life	10	2.9
Health Services Research	1	0.3
Multicenter Research		
Yes	118	34.3
No	218	63.4
Unknown	8	2.3
Recruitment Status		
Not yet recruiting	116	33.7

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Recruiting	171	49.7
Completed	38	11.1
Suspended, Terminated or Withdrawn	19	5.6
Suspended Research	32	9.3
Estimated Enrollment		
Median (Interquartile Range)	120 (180)	
Min, Max	12, 29000	
Missing	3	0.9
Number of Arms		
2 Arms	248	72.1
3 Arms	46	13.4
4 Arms	22	6.4
5 Arms	4	1.2
6-10 Arms	15	4.4
More than 10 Arms	9	2.7
Participants		
Patient	276	80.2
Suspected or Close Contact of Confirmed Patient	7	2.0
Healthy	61	17.7
Masking		
Yes (If any)	100	29.1

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Open Label	105	30.5
Not Stated	139	40.4
Count of Primary Outcome		
0	3	0.9
1	160	46.5
> 1	181	52.6
Median (Interquartile Range)	2 (2)	
Min, Max	0, 23	
Count of Secondary Outcome		
0	84	24.4
> 0	260	75.6
Median (Interquartile Range)	4 (6)	
Min, Max	0, 32	
Publication		
None	285	82.8
Protocol Only	14	4.1
Report Only	43	12.5
Both Protocol and Report	2	0.6

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Table 2. *Deviations between registration records and publications.*

Domain	Protocol (N=16)	Report (N=39)
Sample size		
Not specified in registry and/or publication	0	0
Deviation (Any)	6 (37.5%)	23 (60.0%)
Deviation >= 1%	5 (31.3%)	23 (60.0%)
Deviation >= 20%	5 (31.3%)	19 (48.7%)
Deviation >= 50%	4 (25%)	13 (33.3%)
Deviation >= 100%	4 (25%)	1 (2.6%)
Setting (Center)		
Not specified in registry and/or publication	3 (18.8%)	12 (30.8%)
Deviation (Any)	6 (37.5%)	10 (25.6%)
Inclusion Criteria		
Not specified in registry and/or publication	0	4 (10.3%)
Deviation (Any)	10 (62.5%)	26 (66.7%)
Exclusion Criteria		
Not specified in registry and/or publication	0	6 (15.4%)
Deviation (Any)	9 (56.3%)	22 (56.4%)

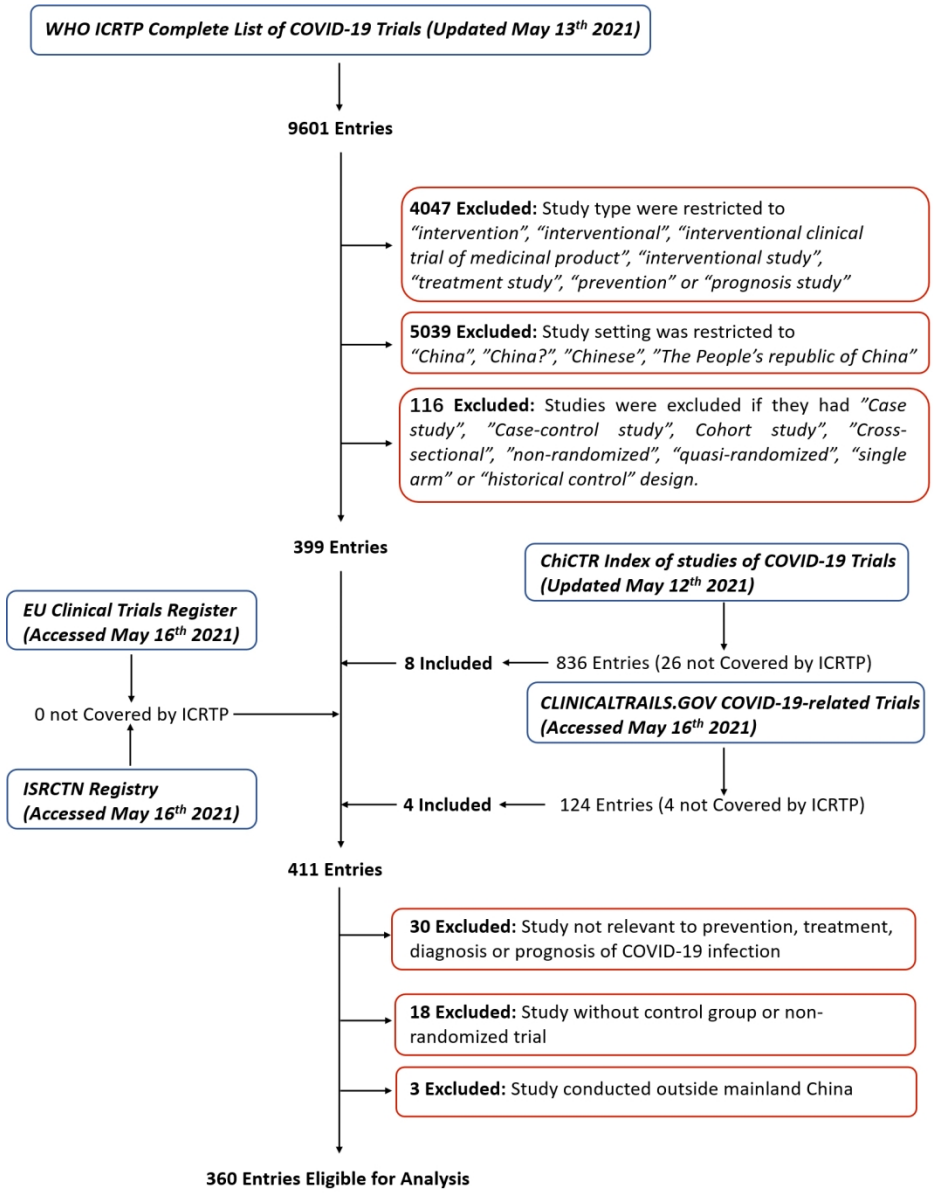
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Masking		
Not specified in registry and/or publication	7 (43.8%)	15 (38.5%)
Deviation (Any)	0	5 (12.8%)
Masking up-graded in publication	0	4 (10.3%)
Masking down-graded in publication	0	1 (2.6%)
Primary Outcome		
Not specified in registry and/or publication	0	2 (5.1%)
Deviation in nature of primary outcome (Any)	8 (50%)	19 (48.7%)
New primary outcome introduced in publication	5 (31.3%)	8 (20.5%)
Registered primary outcome omitted in publication	7 (43.8%)	14 (35.9%)
Secondary outcome in registry described as primary outcome in publication	0	4 (10.3%)
Primary outcome in registry described as secondary outcome in publication	6 (37.5%)	10 (25.6%)
Not specified time frame of primary outcome in registry and/or publication (if no deviation in nature of primary outcome)	1 (6.3%)	2 (5.1%)
Deviation in time frame of primary outcome in registry and/or publication (if no deviation in nature of primary outcome)	1 (6.3%)	4 (10.3%)

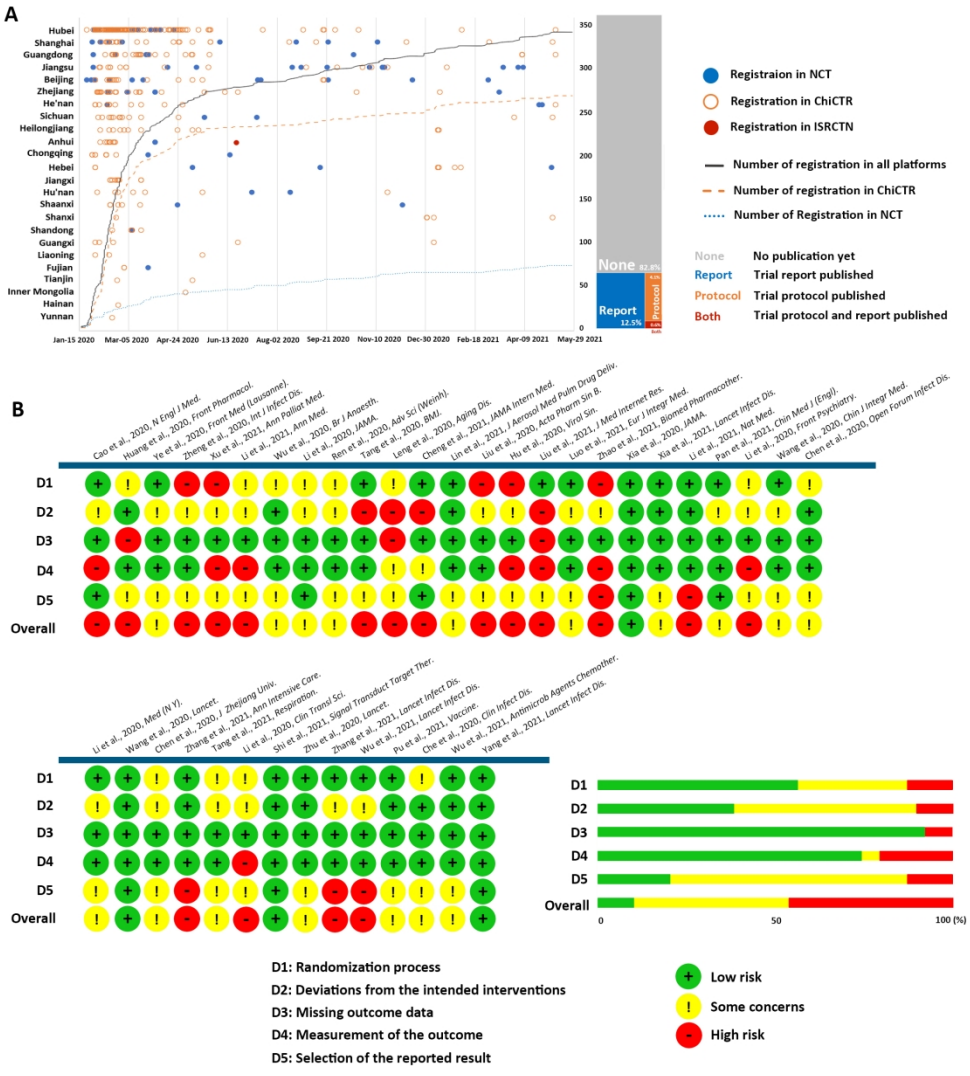
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STROBE Statement—checklist of items that should be included in reports of observational studies

	Item No	Recommendation	Page No
Title and abstract	1	(*a*) Indicate the study's design with a commonly used term in the title or the abstract	3
		(*b*) Provide in the abstract an informative and balanced summary of what was done and what was found	3
Introduction			
Background/rationale	2	Explain the scientific background and rationale for the investigation being reported	6
Objectives	3	State specific objectives, including any prespecified hypotheses	6
Methods			
Study design	4	Present key elements of study design early in the paper	8
Setting	5	Describe the setting, locations, and relevant dates, including periods of recruitment, exposure, follow-up, and data collection	8
Participants	6	(*a*) *Cohort study*—Give the eligibility criteria, and the sources and methods of selection of participants. Describe methods of follow-up *Case-control study*—Give the eligibility criteria, and the sources and methods of case ascertainment and control selection. Give the rationale for the choice of cases and controls *Cross-sectional study*—Give the eligibility criteria, and the sources and methods of selection of participants	8
		(*b*) *Cohort study*—For matched studies, give matching criteria and number of exposed and unexposed *Case-control study*—For matched studies, give matching criteria and the number of controls per case	NA
Variables	7	Clearly define all outcomes, exposures, predictors, potential confounders, and effect modifiers. Give diagnostic criteria, if applicable	9-10
Data sources/ measurement	8*	For each variable of interest, give sources of data and details of methods of assessment (measurement). Describe comparability of assessment methods if there is more than one group	*8*
Bias	9	Describe any efforts to address potential sources of bias	9
Study size	10	Explain how the study size was arrived at	8
Quantitative variables	11	Explain how quantitative variables were handled in the analyses. If applicable, describe which groupings were chosen and why	9-10
Statistical methods	12	(*a*) Describe all statistical methods, including those used to control for confounding	9-10
		(*b*) Describe any methods used to examine subgroups and interactions	NA
		(*c*) Explain how missing data were addressed	NA
		(*d*) *Cohort study*—If applicable, explain how loss to follow-up was addressed *Case-control study*—If applicable, explain how matching of cases and controls was addressed *Cross-sectional study*—If applicable, describe analytical methods taking account of sampling strategy	NA
		(*e*) Describe any sensitivity analyses	NA

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Results			
Participants	13*	(a) Report numbers of individuals at each stage of study—eg numbers potentially eligible, examined for eligibility, confirmed eligible, included in the study, completing follow-up, and analysed	11
		(b) Give reasons for non-participation at each stage	NA
		(c) Consider use of a flow diagram	Fig 1
Descriptive data	14*	(a) Give characteristics of study participants (eg demographic, clinical, social) and information on exposures and potential confounders	Table 1
		(b) Indicate number of participants with missing data for each variable of interest	NA
		(c) *Cohort study*—Summarise follow-up time (eg, average and total amount)	NA
Outcome data	15*	*Cohort study*—Report numbers of outcome events or summary measures over time	NA
		Case-control study—Report numbers in each exposure category, or summary measures of exposure	NA
		Cross-sectional study—Report numbers of outcome events or summary measures	Table 2
Main results	16	(*a*) Give unadjusted estimates and, if applicable, confounder-adjusted estimates and their precision (eg, 95% confidence interval). Make clear which confounders were adjusted for and why they were included	NA
		(*b*) Report category boundaries when continuous variables were categorized	NA
		(*c*) If relevant, consider translating estimates of relative risk into absolute risk for a meaningful time period	NA
Other analyses	17	Report other analyses done—eg analyses of subgroups and interactions, and sensitivity analyses	NA
Discussion			
Key results	18	Summarise key results with reference to study objectives	15
Limitations	19	Discuss limitations of the study, taking into account sources of potential bias or imprecision. Discuss both direction and magnitude of any potential bias	16
Interpretation	20	Give a cautious overall interpretation of results considering objectives, limitations, multiplicity of analyses, results from similar studies, and other relevant evidence	16-17
Generalisability	21	Discuss the generalisability (external validity) of the study results	NA
Other information			
Funding	22	Give the source of funding and the role of the funders for the present study and, if applicable, for the original study on which the present article is based	22

*Give information separately for cases and controls in case-control studies and, if applicable, for exposed and unexposed groups in cohort and cross-sectional studies.

Note: An Explanation and Elaboration article discusses each checklist item and gives methodological background and published examples of transparent reporting. The STROBE checklist is best used in conjunction with this article (freely available on the Web sites of PLoS Medicine at http://www.plosmedicine.org/, Annals of Internal Medicine at http://www.annals.org/, and Epidemiology at http://www.epidem.com/). Information on the STROBE Initiative is available at www.strobe-statement.org.

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BMJ Open

From registration, protocol to report: are COVID-19 related RCTs in mainland China consistent? A systematic review of clinical trial registry and literature

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Keywords:	COVID-19, Clinical trials < THERAPEUTICS, STATISTICS & RESEARCH METHODS

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Title

From registration, protocol to report: are COVID-19 related RCTs in mainland China consistent? A systematic review of clinical trial registry and literature

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Abstract

Objectives

To provide a comprehensive review of registered COVID-19 related RCTs in mainland China and evaluate the transparency of reporting through comparison of registrations, protocols and full reports.

Methods

International Clinical Trials Registry Platform, Chinese Clinical Trial Registry, ClinicalTrials.gov, the ISRCTN registry, and EU Clinical Trial Register were accessed on 1-Feb-2022. Eligible trials were RCTs carried out in mainland China. Studies were excluded if they were observational, non-randomized, or single-arm trials. Publications were searched with trial registration number in PubMed, Embase, Cochrane Library, Google Scholar, CNKI.net and Wanfangdata. Data were extracted from registration records and publications. Risk of bias assessment (RoB2) was performed for trial reports and information provided by registrations and publications was compared. The findings were summarized with descriptive statistics.

Results

The number of eligible studies was 415. From these studies 20 protocols and 77 RCT reports were published. Seven trials published both protocol and RCT full report. Between registrations and publications, discrepancy was found in sample size (9, 39.1% for protocols and 54, 60.0% for reports, same below), trial setting (16, 69.6% and 49, 54.4%), inclusion criteria (15, 65.2% and 67, 74.4%), exclusion criteria (12, 52.2% and 65, 72,2%), masking method (10, 43.5% and 44, 48.9%) and

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primary outcome or time frame of primary outcome measurement (17, 73.9% and 59, 65.6%). Between protocols and full reports, 5 (71.4%) reports had discrepancy in primary outcome or time frame of primary outcome measurement.

Discussion

Discrepancy among registrations, protocols and reports revealed compromised transparency in reporting of COVID-19 related RCTs in mainland China. The importance of trial registration should be further emphasized to enhance transparent RCT reporting.

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Strengths and limitations of this study

- The study provided a full coverage of publicly registered RCTs related to the prevention, treatment or prognosis of COVID-19 infection.
- The study identified publications citing these registration records and examined the consistency of methodology among registrations, protocols and full reports.
- The study included only RCTs performed in mainland China.
- RCT reports that did not cite any registration number (if any) were not included in the analysis.

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Introduction

Evidence-based medicine aims to achieve optimal decision making in the care of individual patients, using the current best evidence[1]. Among all types of evidence, randomized controlled trials (RCT), together with systematic reviews of RCTs, are accorded the highest level of credibility and thus play an important role in evidence-based medicine[2]. However, not all RCTs are of the same quality, which leads to clinical research methodologists' emphasis on transparency in RCT reporting[3].

"The whole of medicine depends on the transparent reporting of clinical trials."[4]

Transparent reporting requires a complete description of methodology through which the trial data are collected and analyzed, a report without omitting any data generated by the trial and a standard way of writing[3]. Much effort has been spent on promoting transparent reporting, including ICMJE member's requirement for registration in public trial registry prior to enrollment[5] and the announcement of CONSORT statement[6]. However, the overall report transparency of RCT remains suboptimal, and incomplete or selective reporting in publication remains an issue. Before mandatory clinical trial registration was enforced by ICMJE in 2005, Chen *et al.* reported 62% published trial report had modified, introduced or omitted in comparing the trial protocols approved by the Scientific-Ethical Committees for Copenhagen and Frederiksberg[7]; when similar approach was applied to trials approved for funding by the Canadian Institutes of Health Research, researchers found that primary outcome differed between reports and protocols in 40% of the trials [8]. To ensure transparency in trial reporting and reduce selective reporting, the ICMJE

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initiative encouraged researchers to make trial information available to the public[5]. Since then, the number of registrations has increased greatly[9,10]. Prospective registration is a powerful tool in reducing selective reporting as they may reflect researchers' intention at planning stage of the trials, and can be compared with published full reports[11]. Mathieu *et al.* compared registrations and publications of randomized controlled trials in journals with highest impact factors in cardiology, rheumatology and gastroenterology, finding that 31% or properly registered RCTs had discrepancies between registered and published primary outcomes[12]; according to Rayhill *et al.*, only about 25% of RCTs published in the core headache medicine journals displayed proper compliance with trial registration[13]. In two systematic reviews summarizing studies that compared registrations with full reports, Jones *et al.* found the median proportion of trials with identified discrepancy in primary outcome was 31%[14], and Li *et al.* also reported high level of inconsistency in outcome reporting ranging from 14% to 100%[11].

The ongoing pandemic of Corona Virus Disease 2019 (COVID-19) is a major public health concern. Numerous clinical trials have been registered to address scientific questions regarding prevention, treatment and prognosis of the disease, and so far, many reports have been published. Kataoka *et al.* have examined COVID-19 RCT articles in medRxiv and PubMed, revealing problems in research methods and the impact on report quality associated with accelerated publication[15]. However, no study has examined the transparency and selective reporting in COVID-19 trials by comparing full reports with trial registrations. In this study, we aimed to provide a comprehensive review of characteristics of registered COVID-19 related RCTs in mainland China, and evaluate

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the level of transparency and selective reporting by comparing trial registrations, published protocols and full reports.

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Methods

Clinical trial registration screening

International Clinical Trials Registry Platform (ICTRP), Chinese Clinical Trial Registry (ChiCTR), ClinicalTrials.gov (NCT), the ISRCTN registry (ISRCTN) and EU Clinical Trial Register (EUCTR) were accessed on 1-Feb-2022. A complete list of COVID-19 trials updated on 22-Jan-2022 was retrieved from ICTRP. Eligible studies were RCTs related to prevention, treatment or prognosis of COVID-19 infection. Studies were excluded if they were observational, non-randomized, single-arm trials or outside of mainland China. For multicenter trials, all centers must be located in mainland China to be eligible for analysis. From ChiCTR, index of studies of COVID-19 updated on 22-Dec-2021 was obtained. Studies registered after this date were screened manually. From NCT, list of interventional clinical studies related to COVID-19 was accessed, and map panel was used to select studies performed in mainland China. For ISRCTN and EUCTR, searches were conducted manually with keywords “COVID-19”, “covid19”, “SARS-Cov-2” or “2019-nCov” and the results were screened for eligibility according to above mentioned criteria (for review protocol and detailed search strategy, see Supplementary 1). From registration records, registration ID, date of registration, date of last update, date of first enrollment, scientific/official title, objective, intervention of interest, comparator, primary purpose, recruitment status, estimated enrollment, arms, ethical approval information (ChiCTR only), name and location of centers, randomization (ChiCTR only), masking, inclusion and exclusion criteria, primary outcomes and other relevant information were

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extracted. Repeated registration was identified through examining similarity of trial characteristics. For multi-registered trials, only the record cited by publication and (if all/none of repeated registration records was cited) the most recently updated record shall be eligible for further analysis. Characteristics of included registrations were summarized and presented as count (%) or median (Interquartile range). Scatter plot and line chart were plotted to illustrate the trend of registered trials in each province of mainland China.

Literature search

Literatures citing these registrations were searched from 10-Feb-2022 to 12-Feb-2022 in PubMed, Embase, Cochrane Library, Google Scholar and two frequently used databases for Chinese language literature, CNKI.net and Wanfangdata, using registration ID as unique identifiers. From published protocols and trial reports, date of first enrollment, estimated enrollment, arms, center names, inclusion/exclusion criteria, primary/secondary outcomes and information regarding randomization and masking were extracted. Risk of bias assessment for published trial reports was performed using RoB 2[16]. Next, information extracted from registration records, published protocols and reports was compared, and the level of consistency was evaluated.

Registration screening, literature search, data extraction, risk of bias assessment and comparison were independently performed by two reviewers and consensus was reached through discussion.

Patient and Public Involvement

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Patients or public was not involved in the design, conduct, reporting, or dissemination plans, since the research question of this study made such involvement unnecessary.

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Results

A total of 435 registration records met eligibility criteria, in which 20 studies were repeatedly registered on ChiCTR and NCT, making the number of eligible studies 415 (see Supplementary 2). The flow chart of screening process is shown in ***Figure 1***.

Characteristics of registered trials

Among all included studies, 303 (73.0%) were registered in ChiCTR, 111 (26.7%) in NCT and 1 (0.2%) in ISRCTN. Only 243 (58.6%) of them were registered before date of first enrollment. Most of the trials aimed to investigate the prevention (105, 25.3%) or treatment (290, 69.9%) of COVID-19. At time of most recent update, 133 (32.0%) trials did not start recruitment, 206 (49.6%) were recruiting and 55 (13.3%) trials completed recruitment, while 21 (5.1%) studies were suspended, terminated or withdrawn. The median of estimated enrollment was 120. 32.8% trials (126) were multicentered, and 68.4% (284) were two-armed. For participants, 285 (68.7%) trials recruited clinical diagnosed COIVD-19 infected patients, 8 (1.9%) recruited suspected cases or close contacts of diagnosed patients, and 122 (29.4%) recruited participants not infected by COVID-19. 39.5% (164) registrations didn't specify any masking method, 125 (30.1%) were open-label studies and single/double masking was applied in 126 (30.4%) studies. The 415 trials registered a total of 1048 primary outcomes. 186 (44.8%) trials specified 1 primary outcome in registration record and 226 (54.5%) declared more than one primary outcomes. Primary outcome was missing for 3 (0.9%) registrations. 313 (75.4%) registrations had pre-specified secondary outcomes. (***Table 1***)

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Charts were plotted to illustrate the trend of increasing trial registration in mainland China and each province. The first COVID-19 trial in mainland China was registered on 23-Jan-2020. As the pandemic began in Wuhan, COVID-19 related RCTs emerged in Hubei province first, and then expanded to other provinces. The next four months witnessed dramatic increase of trial registrations. This trend reached plateau around June 2020 and was thereafter steadily increasing until the day of data extraction (***Figure 2A***).

Publications from registered trials

From the 415 registered trials, 85 reports and 20 protocols were published. 8 reports were excluded from analysis since the study design was described as non-RCT (see Supplementary 1). Risk of bias assessment was performed for 77 RCT reports. Overall, high risk was found in 35 (45.5%) studies; 33 (42,9%) studies had some concerns; 9 (11.7%) were at low risk. (***Figure 2B)***

Comparison between protocols and trial registrations

In 20 published protocols, 6 (30.0%) had deviation in sample size and 8 (40.0%) had discrepancy in setting; More than half of them differed in inclusion criteria (12, 60.0%) or exclusion criteria (10, 50.0%); 9 (45.0%) did not specified masking in text or in registration. In terms of primary outcome, 11 (55%) protocols had different levels of discrepancy, including introducing new primary outcomes in protocol (8, 40.0%), omitting registered primary outcomes (8, 40.0%) and describing registered primary outcomes as secondary outcomes (9, 45.5%). Only 2 (10.0%) protocols kept full consistency in all domains (***Table 2)*** (see Supplementary 2).

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Comparison between reports and trial registrations

Among 77 published RCT reports eligible for analysis, 47 (61.0%) had discrepancies in sample size and the deviation was more than 20% in 36 (46.8%) of them. Discrepancy in setting was present in 21 (27.3%) reports. 54 (70.1%) reports had deviation in inclusion criteria and 46 (59.7%) had different exclusion criteria. 7 (9.1%) reports had deviation in masking method, among which masking was upgraded (open-label changed to single/double-blind, or single-blind changed to double-blind) in 5 (6.5%) studies and downgraded in 2 (2.6%) study. In primary outcome, 4 (5.2%) reports did not specify primary outcome in text or corresponding registration; 34 (44.2%) reports had deviation in primary outcome, including introducing new primary outcomes (18, 23.4%), omitting registered primary outcomes (23, 29.9%), describing registered secondary outcomes as primary outcomes (7, 9.1%) and describing registered primary outcomes as secondary outcomes (15, 19.5%). Furthermore, 7 (9.1%) reports did not specify time frame of primary outcome measurement in registry/text and 6 (7.8%) had deviation in time frame of primary outcome measurement. Only 4 (5.2%) reports maintained full consistency in all domains (***Table 2)*** (see Supplementary 2).

Comparison between protocols and full reports

There were 7 trials where both protocol and RCT report were published. When comparing full reports to protocols, deviation in sample size and setting was found in 6 (85.7%) and 3 (42.9%) reports, respectively. More than half of the reports (4, 57.1%) differed from protocols in inclusion criteria and 3 (42.9%) differed in exclusion criteria.

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Deviation in masking was found in 1 (14.3%) report. 5 (71.4%) reports had difference in primary outcome or time frame of primary outcome. None of the reports maintained full consistency with the protocols (*Table 3*) (see Supplementary 2).

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Discussion

To summarize, 20 protocols and 77 RCT reports were published from 415 registrations in mainland China. Comparing to registrations, 90% protocols and 94.8% reports had discrepancy in at least one domain, in which deviation in primary outcome or time frame of primary outcome measurement occurred in 60% (12/20) protocols and 51.9% (40/77) reports. In trials where both protocol and report were published, 71.4% (5/7) full reports had discrepancies in primary outcome or time frame of measurement comparing to the protocols.

Since the pandemic of COVID-19 is a major public health issue, RCT evidence is urgently needed to support clinical decision making in the prevention, treatment or prognosis of this disease. However, methodological concerns in such trials were noted. Previously, Kataoka *et al.* reported research methods and reporting problems in medRxiv and PubMed publications related to COVID-19 RCTs, and inconsistency with trial registration was identified in 62% of 13 medRxiv literatures and 30% of 16 PubMed articles; the authors pointed out that these problems might result from the accelerated publication process[15], yet the study only covered a limited number of publications because it was performed in June 2020. One year and a half later, our comprehensive review of COVID-19 trial registrations, protocols and reports suggested the finding was not incidental.

To assess the significance of our findings, we compared the results with other studies that evaluated the consistency among trial registrations, protocols and reports. In 2011,

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Dwan *et al.* systematically reviewed cohort studies comparing contents of trial registry entries or protocols with trial reports, and in 33% to 67% of RCTs primary outcome was the same in protocol as in publication, while in 69% to 82% of RCTs primary outcome in report was consistent with trial registration[17]; in the study of Li *et al.*, the median inconsistency of outcome reporting was 54% (IQR: 29% ~ 72%)[11]; Jones *et al.* reported a median proportion of 31% trials had discrepancy between registered and published primary outcome. The rates of discrepancy reported by our study were higher than Dwan *et al.* or Jones *et al.* studies, and were similar to the median rate reported by Li *et al.* These results revealed compromised transparency in COVID-19 related RCT reporting in mainland China, and highlighted the necessity to pay attention to potential selective reporting in not only COVID-19 related RCTs but also RCTs in other fields of study, especially for peer-reviewers and journal editors, as compromised transparency might cast shadow on the value of evidence. Furthermore, the fact that RCT reports were published by less than a quarter of registered trials indicated potential presence of publication bias: a phenomenon of selective publication of studies depending on the results[18]. However, since our research methods were not designed to detect publication bias, we decided to leave this question to future research.

ICMJE's policy of mandatory prospective registration in member journals led to dramatic increase of trial registrations, while the registration data were often inadequate, changed over time and differed between registration and publication[19]. The goal of ICMJE's policy was to promote transparent reporting of trials, yet this could not be achieved unless journal editors and reviewers fully utilize information provided by trial registration: a

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survey by Mathieu *et al.* suggested only around 1/3 reviewers routinely used registration information when evaluating manuscripts[20]. Changes in primary outcome or other domains of trial design might be due to either good reasons or investigators' effort to produce favorable results from the data[14]. In either case, the validity of evidence provided by the trial could not be confidently assessed without emphasizing transparency of reporting. Cooperation of different stakeholders is required to promote transparency. We suggest principal investigators should ensure that the trial is prospectively registered and registration information is properly filled; peer reviewers should routinely use trial registries to assess manuscripts and demand explanation whenever discrepancy occurs; journal editors should prioritize the evaluation of trial registration in peer review process[11,14]. Future studies will be needed to reveal the trend of consistency over time and assess possible improvement caused by increasing scrutiny of peer reviewers, new policy of journals and ascensive familiarity of investigators with trial registration process.

The study had several limitations. First, the complete lists of COVID-19 related clinical trials were retrieved from ICTRP and ChiCTR and filtered by the labels provided by these lists, thus, the completeness and precision of record screening relied on the correctness of these lists. Second, the search of publications for eligible registration record was conducted with trial registration number as unique identifiers; if a publication did not cite any registration number, the article could not be included in analysis. Therefore, the generalizability of our study is restricted to publications which include trial registration identifiers. Third, the reviewers only analyzed the most recent updated version of registration record, without considering the historical changes, yet the validity of

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conclusion of this review is not jeopardized, since this approach tended to overestimate, but not underestimate, the overall transparency, in that registrations are often modified after trial completion to display false consistency with publications[14,19]. Last, our search results were limited to peer-reviewed English and Chinese literatures, and the findings could not be reliably generalized to literatures which were not peer reviewed (for example, manuscripts posted on preprint servers) or studies published in other languages.

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Conclusion

The high rates of discrepancy among registrations, protocols and full reports of COVID-19 related RCTs in mainland China revealed compromised transparency in trial reporting. Investigators, peer reviewers and journal editors should make efforts to improve the utilization of trial registration information and promote transparent reporting.

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Contributorship Statement

Both reviewers (Y.C. and R.Y.) contributed equally in conceptualization, data extraction, analysis, visualization and drafting the manuscript. Both reviewers have read and approved this manuscript.

Data Availability Statement

Not applicable.

Transparency Statement

The authors affirm that the manuscript is an honest, accurate and transparent of the study reported and no important aspect of the study has been omitted.

Competing interest statement

None declared.

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None.

Ethics Approval Statement

Ethics approval from institutional review board was exempted since the study did not involve human participants or animal subjects.

Registration

The systematic review was not prospectively registered.

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Figure Legend

Figure 1. *The screening process of registration records.*

Figure 2. (A) *The trend of registration of COVID-19 related RCT trials in total and in each province. When a trial is multicentered and recruited participants in multiple provinces, the trial is considered to take place in these provinces simultaneously. The publication status of these registered trials is also shown.* ***(B)*** *Results of risk of bias assessment (RoB2) are shown for each RCT report included in this review, the risks in each domain and overall risk are also summarized.*

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Table 1. *Characteristics of all included studies.*

Characteristic	Value	Proportion (%)
Count of Eligible Studies	415	100
Registry		
ChiCTR	303	73.0
Clinicaltrials.gov	111	26.7
ISRCTN	1	0.2
Registration Status		
Prospective	243	58.6
Retrospective	172	41.4
Primary Purpose		
Prevention	105	25.3
Treatment	290	69.9
Prognosis/Quality of Life	19	4.6
Health Services Research	1	0.2
Multicenter Research		
Yes	136	32.8
No	273	65.8
Unknown	6	1.4
Recruitment Status		
Not yet recruiting	133	32.0
Recruiting	206	49.6

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Completed	55	13.3
Suspended, Terminated or Withdrawn	21	5.1
Suspended Research	34	8.2
Estimated Enrollment		
Median (Interquartile Range)	120 (240)	
Min, Max	0, 29000	
Missing	3	0.9
Number of Arms		
2 Arms	284	68.4
3 Arms	59	14.2
4 Arms	31	7.5
5 Arms	5	1.2
6-10 Arms	25	6.0
More than 10 Arms	11	2.7
Participants		
Patient	285	68.7
Suspected or Close Contact of Confirmed Patient	8	1.9
Healthy	122	29.4
Masking		
Single/Double Blind	126	30.4
Open Label	125	30.1
Not Stated	164	39.5

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Count of Primary Outcome		
0	3	0.7
1	186	44.8
2	101	24.3
3	49	11.8
4	28	6.7
5	14	3.4
More than 5 Primary Outcomes	34	8.2
Median (Interquartile Range)	2 (2)	
Min, Max	0, 23	
Total Number of Primary Outcomes	1048	
Count of Secondary Outcome		
0	102	24.6
> 0	313	75.4
Median (Interquartile Range)	4 (6)	
Min, Max	0, 32	
Publication		
None	322	77.6
Protocol Only	13	3.1
Report Only	73	17.6
Both Protocol and Report	7	1.7

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Table 2. Comparison between registration records and publications.

Domain	Protocol N=20	Report N=77
Sample size		
Not specified in registry or publication	1 (5.0%)	0
Deviation (Any)	6 (30.0%)	47 (61.0%)
Deviation >= 1%	5 (25.0%)	47 (61.0%)
Deviation >= 20%	5 (25.0%)	36 (46.8%)
Deviation >= 50%	4 (20.0%)	25 (32.5%)
Deviation >= 100%	4 (20.0%)	2 (2.6%)
Setting (Center)		
Not specified in registry or publication	5 (25.0%)	22 (28.6%)
Deviation (Any)	8 (40.0%)	21 (27.3%)
Inclusion Criteria		
Not specified in registry or publication	0	3 (3.9%)
Deviation (Any)	12 (60.0%)	54 (70.1%)
Exclusion Criteria		
Not specified in registry or publication	0	8 (10.4%)
Deviation (Any)	10 (50.0%)	46 (59.7%)
Masking		
Not specified in registry or publication	9 (45.0%)	28 (36.4%)
Deviation (Any)	0	7 (9.1%)

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Domain	Protocol N=20	Report N=77
Masking up-graded in publication	0	5 (6.5%)
Masking down-graded in publication	0	2 (2.6%)
Primary Outcome		
Not specified in registry or publication	0	4 (5.2%)
Deviation in nature of primary outcome (Any)	11 (55.0%)	34 (44.2%)
New primary outcome introduced in publication	8 (40.0%)	18 (23.4%)
Registered primary outcome omitted in publication	8 (40.0%)	23 (29.9%)
Secondary outcome in registry described as primary outcome in publication	0	7 (9.1%)
Primary outcome in registry described as secondary outcome in publication	9 (45.0%)	15 (19.5%)
Not specified time frame of primary outcome in registry or publication (if no deviation in nature of primary outcome)	2 (10.0%)	7 (9.1%)
Deviation in time frame of primary outcome in registry or publication (if no deviation in nature of primary outcome)	1 (5.0%)	6 (7.8%)

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Table 3. Comparison between protocols and full reports.

Domain	RCT full reports N=7
Sample size	
Not specified in protocol or report	0
Deviation (Any)	6 (85.7%)
Deviation >= 1%	6 (85.7%)
Deviation >= 20%	5 (71.4%)
Deviation >= 50%	3 (42.9%)
Deviation >= 100%	0
Setting (Center)	
Not specified in protocol or report	2 (28.6%)
Deviation (Any)	3 (42.9%)
Inclusion Criteria	
Not specified in protocol or report	0
Deviation (Any)	4 (57.1%)
Exclusion Criteria	
Not specified in protocol or report	0
Deviation (Any)	3 (42.9%)
Masking	
Not specified in protocol or report	1 (14.3%)
Deviation (Any)	1 (14.3%)

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Domain	RCT full reports N=7
Masking up-graded in report	1 (14.3%)
Masking down-graded in report	0
Primary Outcome	
Not specified in protocol or report	0
Deviation in nature of primary outcome (Any)	3 (42.9%)
New primary outcome introduced in report	1 (14.3%)
Registered primary outcome omitted in report	1 (14.3%)
Secondary outcome in protocol described as primary outcome in report	1 (14.3%)
Primary outcome in protocol described as secondary outcome in report	2 (28.6%)
Not specified time frame of primary outcome in protocol or report (if no deviation in nature of primary outcome)	0
Deviation in time frame of primary outcome in protocol or report (if no deviation in nature of primary outcome)	2 (28.6%)

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Title

: From registration, protocol to report: are COVID-19 related RCTs in mainland China consistent? A systematic review of clinical trial registry and literatureDiscrepancies between registration record and publication in COVID-19 related RCTs carried out in China mainland

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Summary boxes

Section 1: What is already known on this topic

Transparent reporting in RCTs is important as RCT-based evidence is most valued in decision making. In 2009, a study showed that among properly registered RCTs in high impact factor journals, 31% trial reports had different levels of discrepancies between registered and reported primary outcomes. To present day, the consistency between registered and published trial information regarding COVID-19 related RCTs conducted in mainland China has not been evaluated.

Section 2: What this study adds

87.5% published protocols and 97.4% reports had different levels of discrepancies (or missing information) between registration and publication. Discrepancies of primary outcome was present in 62.5% published protocols and 69.2% reports. These results highlighted the need to improve reporting transparency in RCTs.

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Abstract

Objectives

To provide a comprehensive review of registered COVID-19 related RCTs in mainland China and evaluate the ~~reporting~~ transparency of ~~COVID-19 related RCTs~~ reporting ~~carried out in mainland China~~ through comparison ~~between~~ of registrations ~~records and~~, protocols and full reports ~~publications~~.

MethodsDesign

~~Complete lists of COVID-19 related trials were obtained from~~ International Clinical Trials Registry Platform ~~(ICTRP) (updated on 13 May 2021),.~~ Chinese Clinical Trial Registry ~~(ChiCTR)~~, ClinicalTrials.gov ~~(NCT)~~, the ISRCTN registry ~~(ISRCTN)~~, and EU Clinical Trial Register ~~(EUCTR)~~ were ~~searched~~accessed ~~for additional COVID-19 related trials~~ on ~~May 16th 2021~~1-Feb-2022. Eligible trials were~~Only~~ RCTs carried out in mainland China ~~were included~~. Studies were excluded if they were observational, non-randomized, or single-arm trials. ~~Corresponding p~~Publications were searched with trial registration number in PubMed, Embase, Cochrane Library, Google Scholar, CNKI.net and Wanfangdata. Data were extracted from registration records and publications. Risk of bias assessment (RoB2) was performed for trial reports and ~~the~~ information provided by registrations and publications was compared ~~for consistency~~. The findings were summarized with descriptive statistics.~~.~~

Results

~~360 registrations were included, in which 16 studies were repeatedly registered, making t~~The number of eligible studies was 415~~344~~. From these studies 20~~16~~

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protocols and 7739 RCT reports were published. Seven trials published both protocol and RCT full report. In RoB2 assessment, overall, high risk was found in 18 (46.2%) studies; 17 (43,6%) studies had some concerns; 4 (10.3%) were at low risk. B14 (87.5%) protocols and 38 (97.4%) reports had different levels of discrepancies (or missing information) between registrations and publications, d. Discrepancyies distributed in multiple domains includingwas found in sample size (96, 3997.15% for protocols and 5423, 60.0% for reports, same below), trial setting (169, 69956.63% and 49, 546.4%), inclusion criteria (150, 652.25% and 6730, 746.49%), exclusion criteria (129, 526.23% and 6528, 721,28%), masking method (107, 43.58% and 4420, 4851.93%) and primary outcome or time frame of primary outcome measurement (170, 7362.95% and 5927, 659.62%). Between protocols and full reports, 5 (71.4%) reports had discrepancy in primary outcome or time frame of primary outcome measurement.

DiscussionConclusions

The high rate of dDiscrepancy between among registration recordss, protocols and trial reports revealed compromised transparency in reporting of COVID-19 relevant related RCTs performed in China mainlandmainland China. The importance of trial registration should be further emphasized to enhance transparent RCT reporting.

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Strengths and limitations of this study

- The study provided a full coverage of publicly registered RCTs related to the prevention, treatment or prognosis of COVID-19 infection.
- The study identified publications citing these registration records and examined the consistency of methodology ~~between~~ among registrations, protocols and full reports ~~and publication~~.
- The study included only RCTs performed in mainland China.
- RCT reports that did not cite any registration number (if any) were not included in the analysis.

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Introduction

~~Presumably, most readers of this article are familiar with the concept of~~ Evidence-~~b~~Based m~~M~~edicine aims to achieve optimal decision making in the care of individual patients, using the current best evidence[1]. Among all types of evidence, ~~On the top hierarchy of evidence stand~~ r~~R~~andomized c~~C~~ontrolled t~~T~~rials (~~and the meta-analyses based on them~~RCT), together with systematic reviews of RCTs, are accorded the highest level of credibility and thus ~~and their importance cannot be overemphasized~~play an important role in evidence-based medicine[2]. However, not all RCTs are of the same quality~~.~~[3], which leads to ~~one important question:~~clinical research methodologists' emphasis on ~~the~~ transparency in RCT reporting[3].

"The whole of medicine depends on the transparent reporting of clinical trials."[4] Transparent reporting requires a complete description of methodology through which the trial data are collected and analyzed, a report without omitting any data generated by the trial and a standard way of writing~~.~~[3]. Much ~~has been done~~effort has been spent on~~to address this problem~~promoting transparent reporting, including ICMJE member's requirement for registration in public trial registry prior to enrollment~~.~~[5] and~~,~~ the announcement of CONSORT statement~~.~~[6]~~, and the mandatory submission of trial protocol enforced by journals with high impact factors~~. However, the overall report transparency of RCT remains suboptimal, and incomplete or selective reporting in publication remains an issue. ~~Given the pivotal role of RCT in evidence-based medicine, transparent reporting has long been a major concern. In the era when~~ Before mandatory clinical trial

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registration was ~~not~~ enforced by ICMJE~~, which was before~~ in 2005, ~~the work by~~ Chen *et al.* reported 62% published trial report had modified, introduced or omitted in comparing the trial protocols approved by the Scientific-Ethical Committees for Copenhagen and Frederiksberg[7]; when similar approach was applied to trials approved for funding by the Canadian Institutes of Health Research, ~~it was~~researchers found that primary outcome differed between reports and protocols in 40% of the trials [8]. ~~Such prevalence of selective reporting was apparently unacceptable.~~ To ensure transparency in trial reporting and reduce selective reporting, the ICMJE initiative encouraged researchers to make trial information available to the public[5]. Since then, the number of registrations has increased greatly[9,10]. Prospective registration is a powerful tool in reducing selective reporting as ~~Registration records are useful information to journal editors, reviewers and researchers because~~ they may reflect ~~the~~ researchers's intention at planning stage of the trials, and ~~each modification is accompanied with a time stamp by trustworthy third-party~~can be compared with published full reports[11]. ~~In most cases this is the only way to obtain validated trial protocol change history for reviewers. Certain authors have taken this advantage in their works:~~ Mathieu *et al.* compared registrations and publications of randomized controlled trials in journals with highest impact factors in cardiology, rheumatology and gastroenterology, finding that 31% or properly registered RCTs had discrepancies between registered and published primary outcomes[12]; according to Rayhill *et al.*, only about ~~a quarter~~25% of RCTs published in the core headache medicine journals displayed proper compliance with trial registration[13]. In two systematic reviews summarizing studies that compared registrations with full reports, Jones *et al.* found the

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median proportion of trials with identified discrepancy in primary outcome was 31%[14], and Li *et al.* also reported high level of inconsistency in outcome reporting ranging from 14% to 100%[11].

The ongoing pandemic of Corona Virus Disease 2019 (COVID-19) is a major public health concern. Numerous clinical trials have been registered to address scientific questions regarding prevention, treatment and prognosis of the disease, and so far, many reports have been published. Kataoka *et al.* have examined COVID-19 RCT articles in medRxiv and PubMed, revealing problems in research methods and the impact on report quality associated with accelerated publication[15]. However, no study has examined the transparency and selective reporting in COVID-19 trials by comparing full reports with trial registrations.

~~In evaluation of transparency, trial registry plays a special role, since other documents submitted to editors are, in methodologists' point of view, subject to modification, whereas registration information, with well documented history versions and time stamp, may reflect trialists' original intention more accurately.~~ In this study, we aimed to provide a comprehensive review of characteristics of registered COVID-19 related RCTs in mainland China, and evaluate the level of transparency and selective reporting by comparing ~~perform analysis on the transparency of COVID-19 related RCTs performed in China mainland based on a comprehensive consideration of~~ trial registrations~~y information,~~, published ~~trial~~ protocols and full reports.

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Methods

Clinical trial registration screening

International Clinical Trials Registry Platform (ICTRP), Chinese Clinical Trial Registry (ChiCTR), ClinicalTrials.gov (NCT), the ISRCTN registry (ISRCTN) and EU Clinical Trial Register (EUCTR) were accessed on 1-FebMay- 16th 20221. A complete list of COVID-19 trials in csv format, which was updated on 22-JanMay- 13th 20221, was retrieved from ICTRP. Filters were applied to restrict study types toEligible studies were interventional studies and studiesRCTs related to prevention, treatment or prognosis of COVID-19 infection. Studies were excluded if they were of observational, design (case studies, case-control studies, cohort studies, cross-sectional studies), non-randomized (including quasi-randomized), without a control groupsingle-arm trials or carried out outside of China mainlandmainland China. For multicenter trials, all centers must be located in China mainlandmainland China to be eligible for analysis. From ChiCTR, iIndex of studies of Novel Coronavirus Pneumonia (COVID-1999) updated on May 12th 2021122-Dec-2021 was obtained. Studies registered after this date were screened manually. From NCT, list of listed interventional clinical studies related to the coronavirus disease (COVID-19) was accessed and study type was restricted to "Interventional (Clinical Trial)", and map panel was used to select studies performed in China mainlandmainland China. For ISRCTN and EUCTR, searches were conducted manually with keywords "COVID-19", "covid19", "SARS-Cov-2" or "2019-nCov" and the results were screened for eligibility according to above mentioned criteria (for review protocol and detailed search

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strategy, see Supplementary 1). From registration records, registration ID, date of registration, date of last update, date of first enrollment, scientific/official title, objective, intervention of interest, comparator, primary purpose, recruitment status, estimated enrollment, arms, ~~registration status (ChiCTR only)~~, ethical approval information (ChiCTR only), name and location of centers, randomization (ChiCTR only), masking, inclusion and exclusion criteria, primary ~~and other~~ outcomes and other relevant information were extracted. ~~For repeated registration records of same trials, information extracted from different registries was compared to determine if discrepancies were present.~~ Repeated registration was identified through examining similarity of ~~title~~trial characteristics. ~~and identity of primary sponsor.~~ For multi-registered trials, only the record cited by publication and (if all/none of repeated registration records was cited) the most recently updated record ~~with most recent update time~~ shall ~~enter~~be eligible for further analysis ~~process~~. Characteristics of included registrations were summarized and presented as count (%) or median (Interquartile range). Scatter plot and line chart were plotted to illustrate the trend of registered trials in each province of mainland China.~~Next, eligible registration records were pooled for analysis.~~

Literature search

L~~Search for publication which cited~~iteratures citing these registration ~~record~~s were searched from~~performed on the same day~~10-Feb-2022 to 12-Feb-2022 in~~at~~ PubMed, Embase, ~~and~~ Cochrane Library, Google Scholar , ~~,~~and two frequently used databases for Chinese language literature, CNKI.net and Wanfangdata, using registration ID as unique

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identifiers. ~~From registration records, registration ID, date of registration, date of last update, date of first enrollment, scientific/official title, objective, intervention of interest, comparator, primary purpose, recruitment status, estimated enrollment, arms, registration status (ChiCTR only), ethical approval information (ChiCTR only), name and location of centers, randomization (ChiCTR only), masking, inclusion and exclusion criteria, primary and other outcomes and other relevant information were extracted.~~ From published protocols and trial reports, date of first enrollment, estimated enrollment, arms, center names, inclusion/exclusion criteria, primary/secondary outcomes and information regarding randomization and masking were extracted. ~~In addition, r~~Risk of bias assessment for published trial report~~cord~~s was performed ~~with~~ using RoB 2[16].

~~Screening, data extraction and risk of bias assessment were independently performed by two reviewers and consensus was reached through discussion.~~

~~For repeated registration records of same trials, information extracted from different registries was compared to determine if discrepancies were present. Repeated registration was identified through similarity of title and identity of primary sponsor. For multi-registered trials, only the record cited by publication and (if all/none of repeated registration records was cited) the record with most recent update time shall enter further analysis process.~~

~~Characteristics of eligible registrations, including trial registry, registration status, primary~~

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~~purpose, recruitment status, if the trial was multicentered or suspended, eligible participants, number of arms, estimated enrollment, number of primary/secondary outcomes, masking status and publication status were summarized and presented as Number (%) or Median (Interquartile Range) depending on categorical or quantitative nature of the data. Scatter plot and line chart were plotted to illustrated the trend of newly registered trials in China mainland and different provinces. Finally~~Next, information extracted from ~~published trial protocols and reports, information regarding estimated enrollment, center name, inclusion/exclusion criteria, masking, randomization, primary/secondary outcome and date of first enrollment (report only) was extracted and compared with~~ registration records, published protocols and reports was compared, ~~among which the~~and the level of consistency was evaluated ~~and summarized~~.

Registration s~~S~~creening, literature search, data extraction, ~~and~~ risk of bias assessment and comparison were independently performed by two reviewers and consensus was reached through discussion.

Patient and Public Involvement

~~Neither p~~Patients ~~n~~or ~~the~~ public was not involved in the design, ~~or~~ conduct, ~~or~~ reporting, or dissemination plans, since the ~~nature~~ research question of this study made ~~these~~ such involvement~~s~~ ~~inappropriate~~unnecessary.

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Results

A total of 435360 registration records met eligibility criteria, in which 2016 studies were repeatedly registered on ChiCTR and NCT, making the number of eligible studies 415 (see Supplementary 2)344. The flow chart of screening process is shown in ***Figure 1***.

Characteristics of registered trials

Among all included studies, 303270 (738.05%) were registered inon ChiCTR, 11173 (261.72%) inon NCT and 1 (0.23%) inon ISRCTN. Only 24305 (589.6%) of them were prospectively registered (defined as registration date prior to or onbefore date of first enrollment) and 139 (40.4%) were retrospectively registered. The primary purposes for most studies wereMost of the trials aimed to investigate the prevention (10560, 2517.34%) or treatment (2980, 6978.95%) of COVID-19. By At time of the time of data extractionmost recent update, 133 (32.0%) trials did not start recruitment, 206 (49.6%) were recruiting and 5538 (131.31%) studies trialshave completed recruitmentenrollment process (recruitment status was completed, no longer recruiting or active, not recruiting), 171 (49.7%) were still recruiting (recruitment status was recruiting or enrolling by invitation) and 116 (33.7%) were not yet recruiting, while 21 (5.1%)19 studies were suspended, terminated or withdrawn. The 118 (34.3%) were multicenter studies (defined as declaration of more than 1 center in registration record), 218 (63.4%) were single center studies and 8 (2.3%) didn't provide sufficient information. Notably, 19 (5.5%) registrations declared to be multicentered in title or other part of registration but had only one or no specified center. mMedian of estimated enrollment was 120, with

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maximum of 29,000 and minimum of 12. 32.8% trials (126) were multicentered, and 68.4% (284)48 (72.1%) were two-armed studies, 46 (13.4%) were three-armed, 22 (6.4%) were four-armed and 28 had five or more than five arms. For participants, As for eligible participants, 28576 (6880.72%) trials recruited clinical diagnosed COIVD-19 infected patients, 87 (12.90%) recruited suspected cases or close contacts of diagnosed patients, while and 12261 (2917.47%) recruited participants not infected by COVID-19. 39.5% (As for masking, 164)39 (40.4%) studies registrations didn't specify any masking method, 1205 (30.15%) were open-label studies and single/double (any kind of) masking was adopted applied in 12600 (3029.41%) studies. Regarding outcomes,The 415 trials registered a total of 1048 primary outcomes. 18660 (446.85%) trials specified 1 primary outcome in registration record and 226181 (542.56%) declared more than one primary outcomes. Primary outcome was missing for 3 (0.9%) trials had missing primary outcomeregistrations. Secondary outcome (if any) was specified in 313260 (75.46%) studiesregistrations had pre-specified secondary outcomes. (The characteristics of included studies based on registration record are summarized in *Table 1*).

Charts were plotted tTo illustrate the trend of increasing trial registration in China mainlandmainland China and each province within, a line and scatter point chart was plotted. As it shows, tThe first COVID-19 trial in mainland China was registered on Jan-23-Jan--2020. As the pandemic began in Wuhan, COVID-19 related RCTs concentrated emerged in Hubei province at first, and then expanded to other provinces. The coming next four months witnessed an explosivedramatic increase in number of newly registered trial registrationss. This trend reached plateau around June 2020 and was thereafter

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steadily increasing until the day of data extraction (***Figure 2A***).

Publications from registered trials

From the 415 registered trials, A total of 8544 reports[17-60] and 2016 protocols[61-76] wereas published from these registered trials, in which. 2 studies published both protocol[64,71] and report[42,51]. Among the reports, 8 reports5 were excluded from analysis since they were case series studies or retrospective studies the study design was described as non-RCT (see Supplementary 1)[56-60]. Risk of bias assessment was performed forin 77 RCTthe remaining 39 reports (***Figure 2B)***. Overall, high risk was found in 3518 (456.52%) studies; 3317 (423,96%) studies had some concerns; 94 (110.73%) were at low risk. (***Figure 2B)***

Comparison between protocols and trial registrations

Next, the consistency between published protocol/report and trial registration information was examined. In 2016 publishedpublished protocols, 6 (307.05%) had deviation in sample size and 5 (31.3%) had more than 20% deviation; 3 (18.8%) had not specified any center in registration or protocol and 86 (4037.05%) had discrepancy in centersetting; More than half of them had deviationdiffered in inclusion criteria (120, 602.05%) or exclusion criteria (109, 506.03%); 97 (453.08%) had did not specified masking in text or in registration.; In terms of primary outcome, 118 (550%) protocols had different levels of discrepancyies, including newly introducedintroducing new primary outcomes in protocol (85, 4031.03%), omittinged registered primary outcomes (87, 4043.08%) and describing registered primary outcomes described as secondary outcomes in text (96, 4537.5%).; Oeonly 2 (102.05%) protocols kept full consistency in all

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~~of the~~ domains ~~–~~(***Table 2)*** (see Supplementary 2).

Comparison between reports and trial registrations

Among 77~~39~~ published RCT reports eligible for analysis, 47~~23~~ (61.0~~0~~%) had discrepancies in sample size and~~of them,~~ the deviation was more than 20% in 36~~19~~ (4~~6~~8.~~8~~7%) of them~~; in one study the deviation was because of reporting of interim analysis~~[46]. ~~Setting was not specified in registry and/or text in 12 (30.8%) reports, and~~ ~~d~~Discrepancy in setting was present in 21~~10~~ (27~~5~~.3~~6~~%) reports. ~~4 (10.3%) and 6 (15.4%) studies had unspecified inclusion criteria or exclusion criteria, respectively, in registry and/or text. In those where eligibility criteria were specified,~~ 54~~26~~ (70~~66~~.1~~7~~%) reports had deviation in inclusion criteria and 46~~22~~ (59~~6~~.7~~4~~%) had different exclusion criteria ~~compared to registration~~. ~~15 (38.5%) had unspecified masking method in registry/text and~~ 7~~5~~ (9~~12~~.1~~8~~%) reports had deviation in masking method, among which masking was upgraded (open-label changed to single/double-blind, or single-blind changed to double-blind) in 5~~4~~ (6~~10~~.5~~3~~%) studies and downgraded in 2~~1~~ (2.6%) study. In ~~respect of~~ primary outcome, 4~~2~~ (5.2~~1~~%) reports ~~had no~~did not specify~~ied~~ primary outcome in text or corresponding registration; 34~~19~~ (44~~48~~.2~~7~~%) reports had deviation in ~~nature of~~ primary outcome, including introduc~~tion of~~ing new primary outcome~~s in text~~ (1~~8~~, 23~~0~~.4~~5~~%), omitting ~~of~~ registered primary outcomes (23~~14~~, 29~~35~~.9%),~~–~~ describing registered secondary outcomes ~~claimed~~ as primary outcome~~s in text~~ (7~~4~~, 9~~10~~.1~~3~~%) and describing registered primary outcomes ~~claimed~~ as secondary outcome~~s intext~~ (1~~50~~, 19~~25~~.5~~6~~%). ~~In studies whose nature of primary outcome was consistent~~Furthermore, 7~~2~~ (9~~5~~.1%) reports

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did not specify time frame of primary outcome measurement in registry/text and 64 (710.83%) had deviation in time frame of primary outcome measurement (*Table 2)*. Only 41 (52.26%) studreportsy had maintained full consistency in above-mentionedall domains between registration and report. (*Table 2)* (see Supplementary 2).

Comparison between protocols and full reports

There were 7 trials where both protocol and RCT report were published. When comparing full reports to protocols, deviation in sample size and setting was found in 6 (85.7%) and 3 (42.9%) reports, respectively. More than half of the reports (4, 57.1%) differed from protocols in inclusion criteria and 3 (42.9%) differed in exclusion criteria. Deviation in masking was found in 1 (14.3%) report. 5 (71.4%) reports had difference in primary outcome or time frame of primary outcome. None of the reports maintained full consistency with the protocols (*Table 3*) (see Supplementary 2).

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Discussion

To summarize, 2016 protocols and 7739 RCT reports were published from 415344 ~~COVID-19 related RCT~~ registrations ~~carried out~~ in mainland China. Comparing to registrations~~Among these publications~~, 9087.5% protocols and 947.84% reports had discrepancy in at least one domain~~ies comparing to registration record~~, in which deviation in primary outcome or time frame of primary outcome measurement occurred in 6062.6% (12/20) protocols and 51.969.2% (40/77) reports~~had unspecified/modified primary outcome or time frame of primary outcome measurement~~. In trials where both protocol and report were published, 71.4% (5/7) full reports had discrepancies in primary outcome or time frame of measurement comparing to the protocols.

Since the pandemic of COVID-19 is a major public health issue, RCT evidence is urgently needed to support clinical decision making in the prevention, treatment or prognosis of this disease. However, methodological concerns in such trials were noted. Previously, Kataoka *et al.* reported research methods and reporting problems in medRxiv and PubMed publications related to COVID-19 RCTs, and inconsistency with trial registration was identified in 62% of 13 medRxiv literatures and 30% of 16 PubMed articles; the authors pointed out that these problems might result from the accelerated publication process[15], yet the study only covered a limited number of publications because it was performed in June 2020. One year and a half later, our comprehensive review of COVID-19 trial registrations, protocols and reports suggested the finding was not incidental.

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To assess the significance of our findings, we compared the results with other studies that evaluated the consistency among trial registrations, protocols and reports. In 2011, Dwan *et al.* systematically reviewed cohort studies comparing contents of trial registry entries or protocols with trial reports, and in 33% to 67% of RCTs primary outcome was the same in protocol as in publication, while in 69% to 82% of RCTs primary outcome in report was consistent with trial registration[17]; in the study of Li *et al.*, the median inconsistency of outcome reporting was 54% (IQR: 29% ~ 72%)[11]; Jones *et al.* reported a median proportion of 31% trials had discrepancy between registered and published primary outcome. The rates of discrepancy reported by our study were higher than Dwan *et al.* or Jones *et al.* studies, and were similar to the median rate reported by Li *et al.* These results revealed compromised transparency in COVID-19 related RCT reporting in mainland China, and highlighted the necessity to pay attention to potential selective reporting in not only COVID-19 related RCTs but also RCTs in other fields of study, especially for peer-reviewers and journal editors, as compromised transparency might cast shadow on the value of evidence. Furthermore, the fact that RCT reports were published by less than a quarter of registered trials indicated potential presence of publication bias: a phenomenon of selective publication of studies depending on the results[18]. However, since our research methods were not designed to detect publication bias, we decided to leave this question to future research.[15]

ICMJE's policy of mandatory prospective registration in member journals led to dramatic increase of trial registrations, while the registration data were often inadequate, changed over time and differed between registration and publication[19]. The goal of ICMJE's policy

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was to promote transparent reporting of trials, yet this could not be achieved unless journal editors and reviewers fully utilize information provided by trial registration: a survey by Mathieu *et al.* suggested only around 1/3 reviewers routinely used registration information when evaluating manuscripts[20]. Changes in primary outcome or other domains of trial design might be due to either good reasons or investigators' effort to produce favorable results from the data[14]. In either case, the validity of evidence provided by the trial could not be confidently assessed without emphasizing transparency of reporting. Cooperation of different stakeholders is required to promote transparency. We suggest principal investigators should ensure that the trial is prospectively registered and registration information is properly filled; peer reviewers should routinely use trial registries to assess manuscripts and demand explanation whenever discrepancy occurs; journal editors should prioritize the evaluation of trial registration in peer review process[11,14]. Future studies will be needed to reveal the trend of consistency over time and assess possible improvement caused by increasing scrutiny of peer reviewers, new policy of journals and ascensive familiarity of investigators with trial registration process.

The study had several limitations. First, the complete lists of COVID-19 related clinical trials were retrieved from ICTRP and ChiCTR and filtered by the labels provided by these lists, thus, the completeness and precision of record screening relied on the correctness of these lists. Second, the search of publications for eligible registration record was conducted with trial registration number as unique identifiers; if a publication did not cite any registration number, the article could not be included in analysis. Therefore, the generalizability of our study is restricted to publications which include trial registration

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identifiers. Third, the reviewers only analyzed the most recent updated version of registration record, without considering the historical changes, yet the validity of conclusion of this review is not jeopardized, since this approach tended to overestimate, but not underestimate, the overall transparency, in that registrations are often modified after trial completion to display false consistency with publications[14,19]. Last, our search results were limited to peer-reviewed English and Chinese literatures, and the findings could not be reliably generalized to literatures which were not peer reviewed (for example, manuscripts posted on preprint servers) or studies published in other languages.

~~These numbers revealed compromised transparency in RCT reporting.~~

~~Given the pivotal role of RCT in evidence-based medicine, transparent reporting has long been a major concern. In the era when mandatory clinical trial registration was not enforced by ICMJE, which was before 2005, the work by Chen et al. reported 62% published trial report had modified, introduced or omitted in comparing the trial protocols approved by the Scientific-Ethical Committees for Copenhagen and Frederiksberg[7]; when similar approach was applied to trials approved for funding by the Canadian Institutes of Health Research, it was found that primary outcome differed between reports and protocols in 40% of the trials[8]. Such prevalence of selective reporting was apparently unacceptable. To ensure transparency in trial reporting and reduce selective reporting, the ICMJE initiative encouraged researchers to make trial information available~~

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~~to the public[5]. Since then, the number of registrations has increased greatly[9,10]. Registration records are useful information to journal editors, reviewers and researchers because they may reflect the researcher's intention at planning stage of the trials, and each modification is accompanied with a time stamp by trustworthy third-party. In most cases this is the only way to obtain validated trial protocol change history for reviewers. Certain authors have taken this advantage in their works: Mathieu et al. compared registrations and publications of randomized controlled trials in journals with highest impact factors in cardiology, rheumatology and gastroenterology, finding that 31% or properly registered RCTs had discrepancies between registered and published primary outcomes[12]; according to Rayhill et al., only about a quarter of RCTs published in the core headache medicine journals displayed proper compliance with trial registration[13]. Similarly, our results revealed lack of compliance in primary outcome of 62.6% protocols and 69.2% reports in COVID-19 related RCTs. These results highlighted the need to apply more careful examination to potentially underlying outcome reporting bias, especially for peer-reviewers and journal editors. Meanwhile, it is also important to emphasize all required information by trial registry be properly filled, since missing or unspecified information will not enable further evaluation, thus decreasing overall transparency of reporting. Since the pandemic of COVID is a major public health issue, first-hand evidence is urgently needed to support clinical decision making in the prevention, treatment or prognosis of this disease. However, the compromised transparency of COVID-19 related RCT reporting in Mainland China cast shadow on the value of these evidence.~~

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The possible limitations of this study included: First, the complete lists of COVID-19 related clinical trials were retrieved from ICTRP and ChiCTR and filtered by the labels provided by these lists, thus, the completeness and precision of record screening relied on the correctness of these lists; Second, the search of publications for eligible registration record is conducted with trial registration number as unique identifiers, if a publication did not cite any registration number, the article could not be included in analysis. However, considering the requirement for RCT registration is common in journals, we considered these publications to be of a small number (if existed at all). Third, the reviewers only analyzed the most recent updated version of registration record, without considering the historical changes, yet the validity of conclusion of this review is not jeopardized, since this approach tended to overestimate, but not underestimate, the overall transparency.

To emphasize the transparency of RCT reporting and the validity of evidence, cooperation of different stakeholders is required. Principal investigators have the responsibility to ensure that the trial has been registered before first participant is enrolled and that registration record is updated timely whenever the trial protocol is modified; Journal editors and peer reviewers need to carefully check submitted manuscripts and corresponding registration information, demanding explanation whenever discrepancy occurs[11,14]. If the significance of trial registration can be properly

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~~emphasized, we are one more step closer to the goal of full transparent reporting of RCTs.~~

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Conclusion

The high rates of ~~deviation~~ discrepancy~~rate between~~ among registrations, protocol~~and publication~~s and full reports of COVID-19 related RCTs ~~conducted~~ in ~~China mainland~~mainland China revealed compromised transparency in trial reporting. Investigators, peer reviewers and journal editors should make efforts to improve the utilization of trial registration information and promote transparent reporting. ~~The importance for investigators to maintain a timely and detailed registration and for reviewers to routinely scrutinize registration information need to be emphasized.~~

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Contributorship Statement

Both reviewers (Y.C. and R.Y.) contributed equally in conceptualization, data extraction, analysis, visualization and drafting the manuscript. Both reviewers have read and approved this manuscript.

Data AvailabilitySharing Statement

Not applicable.No additional data available.

Transparency Statement

The authors affirm that the manuscript is an honest, accurate and transparent of the study reported and no important aspect of the study has been omitted.

Competing interest statement

None declared.

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Ethics Approval Statement

Ethics approval from institutional review board was exempted since the study did not involve human participants or animal subjects.

Registration

The systematic review was not prospectively registered.

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Figure Legend

Figure 1. *T~~he flow chart shows t~~he screening process o~~ffor eligible~~ registration records.*

Figure 2. (A) *The trend of registration of COVID-19 related RCT trials in total and in each province. When a trial is multicentered and recruited participants in multiple provinces, the trial is considered to take place in these provinces simultaneously. The publication status of these registered trials is also shown.* ***(B)*** *Results of risk of bias assessment (RoB2) are shown for each RCT report included in this review, the risks in each domain and overall risk are also summarized.*

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Table 1. *Characteristics of all included studies.*

Characteristic	Value	Proportion (%)
Count of Eligible Studies	415~~344~~	100
Registry		
ChiCTR	303~~270~~	73~~8~~.0~~5~~
Clinicaltrials.gov	111~~73~~	26~~1~~.7~~2~~
ISRCTN	1	0.2~~3~~
Registration Status		
Prospective	243~~205~~	5~~89~~.6
Retrospective	172~~139~~	4~~10~~.4
Primary Purpose		
Prevention	105~~60~~	25~~17~~.3~~4~~
Treatment	290~~70~~	69~~78~~.9~~5~~
Prognosis/Quality of Life	19~~3~~	4~~0~~.6~~9~~
~~*Quality of Life*~~	~~10~~	~~2.9~~
Health Services Research	1	0.2~~3~~
Multicenter Research		
Yes	136~~18~~	32~~4~~.8~~3~~
No	273~~18~~	65~~3~~.8~~4~~
Unknown	6~~8~~	1~~2~~.4~~3~~
Recruitment Status		
Not yet recruiting	133~~16~~	32~~3~~.0~~7~~

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Recruiting	206171	49.67
Completed	5538	131.31
Suspended, Terminated or Withdrawn	2119	5.16
Suspended Research	342	89.23
Estimated Enrollment		
Median (Interquartile Range)	120 (240180)	
Min, Max	012, 29000	
Missing	3	0.9
Number of Arms		
2 Arms	28448	6872.41
3 Arms	5946	143.24
4 Arms	3122	76.54
5 Arms	54	1.2
6-10 Arms	2515	64.04
More than 10 Arms	119	2.7
Participants		
Patient	28576	6880.72
Suspected or Close Contact of Confirmed Patient	87	12.90
Healthy	12261	2917.47
Masking		

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Single/Double BlindYes (If any)	12600	3029.41
Open Label	1205	30.15
Not Stated	16439	3490.54
Count of Primary Outcome		
0	3	0.79
1	18660	446.85
2> 1	101181	24.352.6
3	49	11.8
4	28	6.7
5	14	3.4
More than 5 Primary Outcomes	34	8.2
Median (Interquartile Range)	2 (2)	
Min, Max	0, 23	
Total Number of Primary Outcomes	1048	
Count of Secondary Outcome		
0	10284	24.64
> 0	313260	75.46
Median (Interquartile Range)	4 (6)	
Min, Max	0, 32	
Publication		
None	322285	7782.68
Protocol Only	134	34.11

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Report Only	7343	1712.65
Both Protocol and Report	72	10.76

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Table 2. ~~Deviations~~ Comparison between registration records and publications.

Domain	Protocol ~~(N=16)~~N=20	Report ~~(N=39)~~ N=77
Sample size		
Not specified in registry ~~and/or~~or publication	1 (5.0%)~~0~~	0
Deviation (Any)	6 (3~~0~~7.0~~5~~%)	47~~23~~ (6~~1~~0.0%)
Deviation >= 1%	5 (25~~31~~.0~~3~~%)	47~~23~~ (6~~1~~0.0%)
Deviation >= 20%	5 (25~~31~~.0~~3~~%)	36~~19~~ (4~~6~~8.~~8~~7%)
Deviation >= 50%	4 (20.0~~5~~%)	25~~13~~ (32~~3~~.5~~3~~%)
Deviation >= 100%	4 (20.0~~5~~%)	2~~1~~ (2.6%)
Setting (Center)		
Not specified in registry ~~and/or~~or publication	5~~3~~ (25~~1~~8.0~~8~~%)	2~~1~~2 (28~~3~~0.6~~8~~%)
Deviation (Any)	8~~6~~ (40~~3~~7.0~~5~~%)	21~~10~~ (27~~5~~.3~~6~~%)
Inclusion Criteria		
Not specified in registry ~~and/or~~or publication	0	3~~4~~ (3~~10~~.9~~3~~%)
Deviation (Any)	1~~2~~0 (60~~2~~.0~~5~~%)	54~~26~~ (70~~6~~6.1~~7~~%)
Exclusion Criteria		

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Domain	Protocol (N=16)N=20	Report (N=39) N=77
Not specified in registry and/oror publication	0	86 (105.4%)
Deviation (Any)	109 (506.03%)	4622 (596.74%)
Masking		
Not specified in registry and/oror publication	97 (453.08%)	2815 (368.45%)
Deviation (Any)	0	75 (912.18%)
Masking up-graded in publication	0	54 (610.53%)
Masking down-graded in publication	0	21 (2.6%)
Primary Outcome		
Not specified in registry and/oror publication	0	42 (5.21%)
Deviation in nature of primary outcome (Any)	118 (55.00%)	3419 (448.27%)
New primary outcome introduced in publication	85 (4031.03%)	188 (230.45%)
Registered primary outcome omitted in publication	87 (4043.08%)	2314 (2935.9%)
Secondary outcome in registry described as primary outcome in publication	0	74 (910.13%)
Primary outcome in registry described as secondary outcome in publication	96 (4537.05%)	1510 (1925.56%)
Not specified time frame of primary outcome in registry and/oror publication (if no deviation in	21 (106.03%)	72 (9.5.1%)

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Domain	Protocol ~~(N=16)~~N=20	Report ~~(N=39)~~ N=77
nature of primary outcome)		
Deviation in time frame of primary outcome in registry ~~and/or~~or publication (if no deviation in nature of primary outcome)	1 (~~56~~.~~03~~%)	~~64~~ (~~710~~.~~83~~%)

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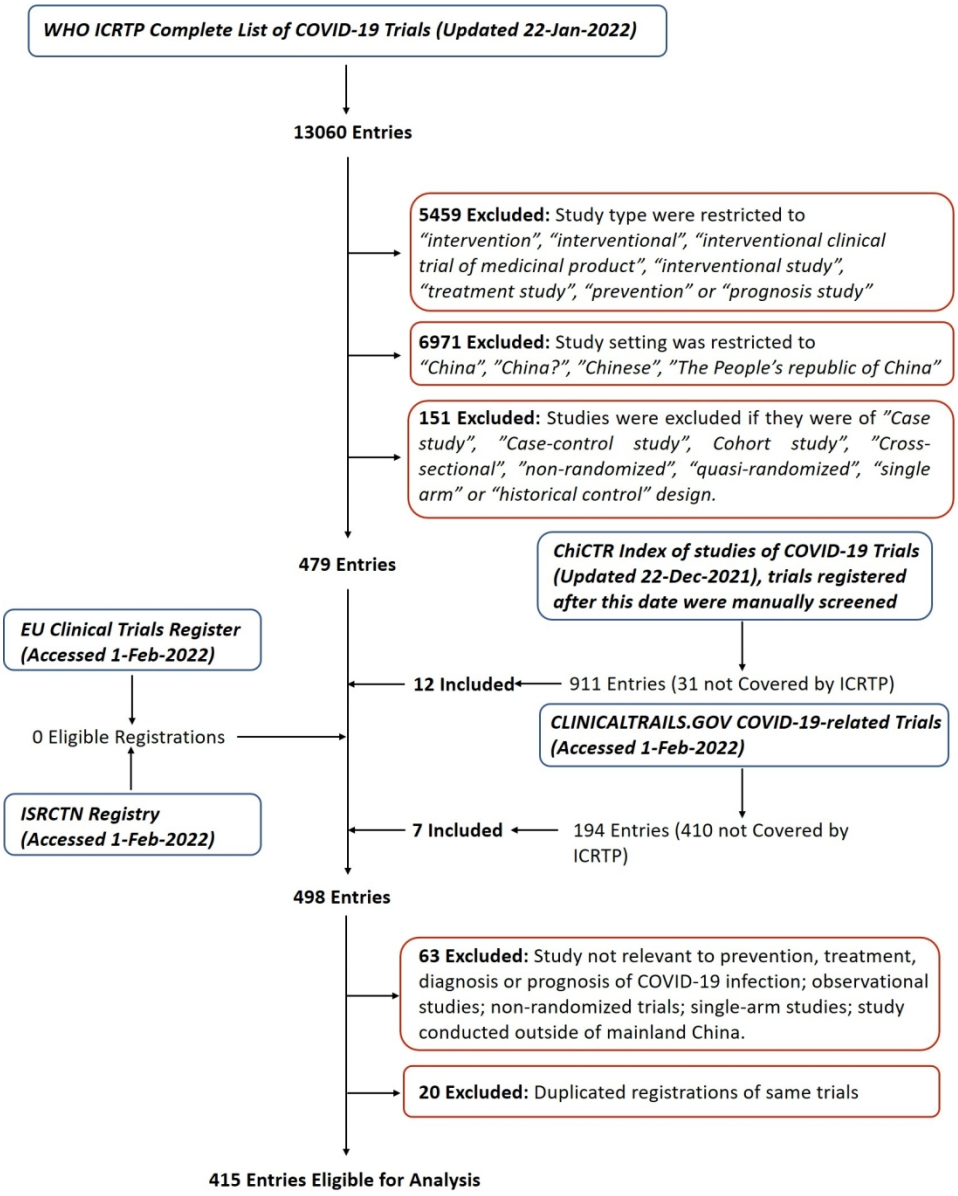
Table 3. Comparison between protocols and full reports.

Domain	RCT full reports N=7
Sample size	
Not specified in protocol or report	0
Deviation (Any)	6 (85.7%)
Deviation >= 1%	6 (85.7%)
Deviation >= 20%	5 (71.4%)
Deviation >= 50%	3 (42.9%)
Deviation >= 100%	0
Setting (Center)	
Not specified in protocol or report	2 (28.6%)
Deviation (Any)	3 (42.9%)
Inclusion Criteria	
Not specified in protocol or report	0
Deviation (Any)	4 (57.1%)
Exclusion Criteria	
Not specified in protocol or report	0
Deviation (Any)	3 (42.9%)
Masking	
Not specified in protocol or report	1 (14.3%)
Deviation (Any)	1 (14.3%)

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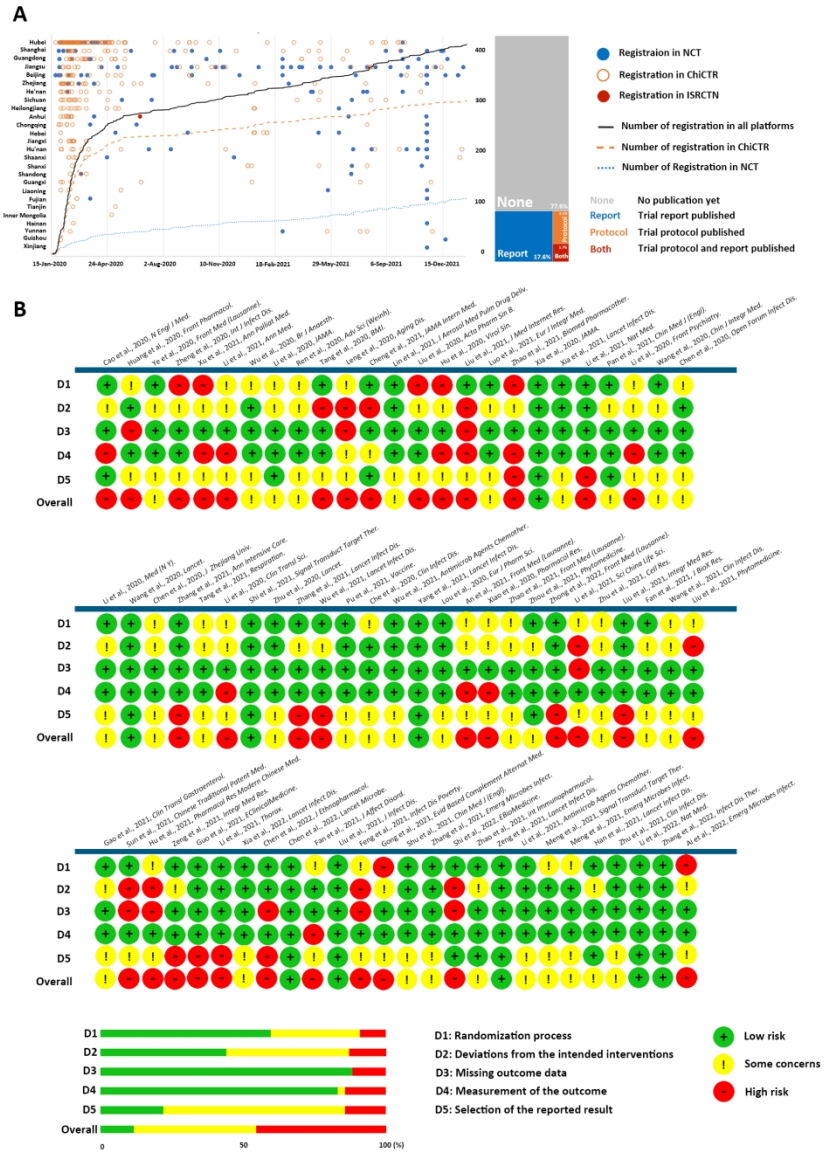
Domain	RCT full reports N=7
Masking up-graded in report	1 (14.3%)
Masking down-graded in report	0
Primary Outcome	
Not specified in protocol or report	0
Deviation in nature of primary outcome (Any)	3 (42.9%)
New primary outcome introduced in report	1 (14.3%)
Registered primary outcome omitted in report	1 (14.3%)
Secondary outcome in protocol described as primary outcome in report	1 (14.3%)
Primary outcome in protocol described as secondary outcome in report	2 (28.6%)
Not specified time frame of primary outcome in protocol or report (if no deviation in nature of primary outcome)	0
Deviation in time frame of primary outcome in protocol or report (if no deviation in nature of primary outcome)	2 (28.6%)

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The screening process of registration records.

290x359mm (150 x 150 DPI)



(A) The trend of registration of COVID-19 related RCT trials in total and in each province. When a trial is multicentered and recruited participants in multiple provinces, the trial is considered to take place in these provinces simultaneously. The publication status of these registered trials is also shown. (B) Results of risk of bias assessment (RoB2) are shown for each RCT report included in this review, the risks in each domain and overall risk are also summarized.

687x978mm (72 x 72 DPI)

Review Protocol

Objective

To evaluate the transparency of COVID-19 related RCT reporting in mainland China through comparing trial registrations with publications.

Ethics Approval

The study is exempt from ethics approval because only publicly available databases and registries will be used as data source. No human participants and animal subjects will be involved in the study.

Eligibility Criteria for Registrations

Registered randomized controlled trials related to prevention, treatment or prognosis of COVID-19 in mainland China will be included. Case report, case series, cross-sectional study, case-control study, cohort study, survey and other observational studies will be excluded. Studies will be excluded if randomization is not used or without a control group. For multicenter trials, all registered centers must be within mainland China to meet eligibility criteria.

Data Source and Search Strategy for Registrations

International Clinical Trials Registry Platform (ICTRP), Chinese Clinical Trial Registry (ChiCTR), ClinicalTrials.gov (NCT), the ISRCTN registry (ISRCTN) and EU Clinical Trial Register (EUCTR) will be searched. For ICTRP and ChiCTR, complete lists of COVID-19 clinical trial registrations will be downloaded. Filters will be applied to identify studies which meet eligibility criteria. For NCT, the page listing COVID-19 studies will be accessed and eligible studies will be identified using website filters and the map panel. ISRTCN and EUCTR will be manually searched with the keywords “COVID-19”, “Sars-Cov-2”, “covid19” and “2019-nCov”. All search results will be examined manually to ensure their eligibility.

Data Source and Search Strategy for Publications

PubMed, Embase, Cochrane Library, CNKI.net and Wanfangdata will be searched using trial

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registration IDs of eligible trials. Only publications in English and Chinese will be included.

Data Extraction

From trial registrations, registration ID, date of registration/first submission, date of last update, date of first enrollment, scientific/official titles, primary purpose, recruitment status, intervention, source of funding, primary sponsor, ethics approval information, setting, randomization and masking methods, inclusion and exclusion criteria, primary/secondary outcomes and time frame of outcome measurement will be extracted.

From publications, title, estimated/actual enrollment, center name, inclusion/exclusion criteria, masking, primary outcomes and time frame of outcome measurement will be extracted.

Risk of Bias Assessment

Risk of bias assessment will be performed with RoB 2 for full reports of trials.

Data Synthesis

The screening process of registrations will be presented with a flow diagram. Characteristics of included trial registrations will be presented with descriptive statistics, in count and proportion for categorical data, or with median, max value, minimum value and interquartile range for quantitative data. The trend of registrations from early 2020 will be presented with line chart. Estimated/actual enrollment, center name, inclusion/exclusion criteria, masking method, primary outcome and time frame of primary outcome measurement information extracted from trial registrations and publications will be compared, and count and percentage of inconsistency within each domain will be presented. Risk of bias assessment results will be presented with figure.

Review Process

The screening process, data extraction, risk of bias assessment, registration-publication comparison will be independently completed by two reviewers, and the results will be compared. Disagreements will be resolved through discussion.

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Protocol Amendments

May-2021

For trials which are repeatedly registered on two or more clinical trial registries, if they have any publication, the registration not cited by publication will be excluded from the analysis. If all or none of the repeated registrations are cited by publications, the record with most recent update time will be included in the analysis. Repeated registrations will be detected by reviewing the title, objective and name of principle investigator.

Publications citing an eligible trial registration identifier but declared to be of non-RCT design will be excluded from the analysis. The number of such publications will be reported.

Feb-2022

Google Scholar will be searched for publications to ensure the completeness of search results.

Information extracted from protocols and corresponding full reports will also be compared to evaluate protocol-report consistency.

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Search Strategy

Clinical Trial Registration

International Clinical Trials Registry Platform (ICTRP, https://www.who.int/clinical-trials-registry-platform) was accessed and a list of COVID-19 trials (updated on 22-Jan-2022) in csv format was downloaded. The file was opened with Microsoft Excel. The following filters were applied to "Study type" column: "intervention" "interventional" "interventional clinical trial of medicinal product" "interventional study" "treatment study" "prevention" "prognosis study". The following filters were applied to "Countries" column: "China" "China?" "Chinese" "The People's republic of China". From the "Study design" column, "Case study" "Case-control study" "Cohort study" "Cross-sectional" studies, non-randomized/quasi-randomized studies and studies with single arm or historical control were excluded.

Chinese Clinical Trial Registry (ChiCTR, http://www.chictr.org.cn/enIndex.aspx) was accessed and index of studies of COVID-19 (updated on 22-Dec-2021) in csv format was downloaded. The ChiCTR index was then mapped to the ICTRP COVID-19 trials list to identify any studies listed in ChiCTR but not in ICTRP. Studies registered after 22-Dec-2021 were screened manually.

List of COVID-19 related studies from ClinicalTrials.gov was accessed (https://clinicaltrials.gov/ct2/results?cond=COVID-19). The filter "Study type - Interventional (Clinical Trial)" was applied, and studies registered in mainland China were identified using the "On Map" panel. The listed studies were downloaded and compared with ICTRP records in case of omissions.

In ISRCTN registry (ISRCTN, https://www.isrctn.com/) and EU Clinical Trial Register (EUCTR, https://www.clinicaltrialsregister.eu/), the registry was searched with the following keywords: "covid19" "COVID-19" "SARS-Cov-2" or "2019-nCov", and the country of recruitment was set to "China" and "Outside EU/EEA", respectively. Search results were also compared with ICTRP records.

All above-mentioned registries were accessed on 1-Feb-2022.

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Publication

Search was performed in PubMed (https://pubmed.ncbi.nlm.nih.gov/), Embase (https://www.embase.com/), Cochrane Library (https://www.cochranelibrary.com/search), Google Scholar (https://scholar.google.com/), CNKI.net (https://www.cnki.net/) and Wanfangdata (https://www.wanfangdata.com.cn/) using the trial registration number with exact match method.

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Literatures

This systematic review identified 85 reports[1-85] and 20[86-105] protocols from 415 clinical trial registration records. For further analysis, 8 reports[78-85] were excluded because non-RCT study design was adopted.

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2. Che Y, Liu X, Pu Y, et al. Randomized, double-blinded and placebo-controlled phase II trial of an inactivated SARS-CoV-2 vaccine in healthy adults. *Clin Infect Dis.* 2020.
3. Chen J, Liu D, Liu L, et al. A Pilot Study of Hydroxychloroquine in Treatment of Patients with Moderate COVID-19. *Journal of Zhejiang University (Medical Sciences).* 2020;49(2).
4. Chen J, Xia L, Liu L, et al. Antiviral Activity and Safety of Darunavir/Cobicistat for the Treatment of COVID-19. *Open Forum Infect Dis.* 2020;7(7):ofaa241.
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6. Hu K, Wang M, Zhao Y, et al. A Small-Scale Medication of Leflunomide as a Treatment of COVID-19 in an Open-Label Blank-Controlled Clinical Trial. *Virol Sin.* 2020;35(6):725-733.
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11. Li J, Li X, Jiang J, et al. The Effect of Cognitive Behavioral Therapy on Depression, Anxiety, and Stress in Patients With COVID-19: A Randomized Controlled Trial. *Front Psychiatry.* 2020;11:580827.
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13. Li T, Sun L, Zhang W, et al. Bromhexine Hydrochloride Tablets for the Treatment of Moderate COVID-19: An Open-Label Randomized Controlled Pilot Study. *Clin Transl Sci.* 2020;13(6):1096-1102.
14. Li Y, Xie Z, Lin W, et al. Efficacy and Safety of Lopinavir/Ritonavir or Arbidol in Adult Patients with Mild/Moderate COVID-19: An Exploratory Randomized Controlled Trial.

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16. Liu X, Li Z, Liu S, et al. Potential therapeutic effects of dipyridamole in the severely ill patients with COVID-19. *Acta Pharm Sin B.* 2020;10(7):1205-1215.
17. Liu Z, Qiao D, Xu Y, et al. The Efficacy of Computerized Cognitive Behavioral Therapy for Depressive and Anxiety Symptoms in Patients With COVID-19: Randomized Controlled Trial. *J Med Internet Res.* 2021;23(5):e26883.
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double-blind, placebo-controlled, phase 1/2 clinical trial. *The Lancet Infectious Diseases.* 2021;21(6):803-812.

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ChiCTR2100053551

NCT05107557

ChiCTR2100049744

ChiCTR2100045647

NCT05112913

NCT05148949

NCT05165732

NCT05165966 ChiCTR2100052697

NCT05172141

ChiCTR2100046612

For peer review only

NCT05167279

NCT05194033

NCT05195047

ChiCTR2100050587

ChiCTR2100050849

ChiCTR2100051998

ChiCTR2100053701

ChiCTR2100053822

ChiCTR2100054324

NCT05204589

For peer review only

NCT05197179

NCT05095298 Report-171

NCT05148091

NCT05184062

ChiCTR2200055638

ChiCTR2200055774

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Non-RCT Publications	Registry Platform	Date of Registration/Fi	Last updated post
	ChiCTR	2020/1/23	3/9/2020
	ClinicalTrials.gov	2020/1/23	6/16/2020
	ClinicalTrials.gov	2020/1/28	5/5/2020
	ChiCTR	2020/1/29	3/2/2020
	ChiCTR	2020/1/29	3/2/2020
	ClinicalTrials.gov	2020/1/29	4/13/2020
	ChiCTR	2020/1/30	2/12/2020
	ClinicalTrials.gov	2020/1/30	3/17/2020
	ClinicalTrials.gov	2020/1/30	7/1/2020

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ClinicalTrials.gov	2020/1/31	4/15/2020
ClinicalTrials.gov	2020/1/31	4/15/2020
ChiCTR	2020/2/1	2/12/2020
ChiCTR	2020/2/1	4/17/2020
ChiCTR	2020/2/1	8/8/2021
ChiCTR	2020/2/1	2/12/2020
ChiCTR	2020/2/1	2/12/2020
ChiCTR	2020/2/1	2/12/2020
ChiCTR	2020/2/2	2/12/2020
ChiCTR	2020/2/2	2/12/2020

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ChiCTR	2020/2/2	2/12/2020
ChiCTR	2020/2/2	2/12/2020
ChiCTR	2020/2/2	2/12/2020
ClinicalTrials.gov	2020/2/2	3/17/2020
ClinicalTrials.gov	2020/2/2	3/17/2020
ClinicalTrials.gov	2020/2/2	3/17/2020
ChiCTR	2020/2/3	3/22/2020
ChiCTR	2020/2/3	2/12/2020
ChiCTR	2020/2/3	2/12/2020
ChiCTR	2020/2/3	2/12/2020

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ChiCTR	2020/2/3	2/12/2020
ChiCTR	2020/2/3	2/12/2020
ChiCTR	2020/2/4	2/12/2020
ChiCTR	2020/2/4	3/9/2020
ChiCTR	2020/2/4	2/12/2020
ChiCTR	2020/2/4	9/29/2020
ClinicalTrials.gov	2020/2/4	3/17/2020
ClinicalTrials.gov	2020/2/4	10/12/2020
ChiCTR	2020/2/5	9/29/2020
ChiCTR	2020/2/5	3/22/2020

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	ChiCTR	2020/2/5	3/9/2020
	ChiCTR	2020/2/5	6/28/2020
Report-906	ChiCTR	2020/2/6	2/12/2020
	ChiCTR	2020/2/6	2/12/2020
	ChiCTR	2020/2/6	2/12/2020
	ChiCTR	2020/2/6	5/15/2020
	ClinicalTrials.gov	2020/2/6	4/13/2020
Report-901-1 Report-901-2	ChiCTR	2020/2/7	2/12/2020
	ChiCTR	2020/2/8	5/16/2020
	ChiCTR	2020/2/9	2/12/2020

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ChiCTR	2020/2/9	3/17/2020
ChiCTR	2020/2/11	2/24/2020
ChiCTR	2020/2/11	2/13/2020
ChiCTR	2020/2/11	2/17/2020
ChiCTR	2020/2/11	2/13/2020
ChiCTR	2020/2/12	3/2/2020
ChiCTR	2020/2/12	2/12/2020
ChiCTR	2020/2/12	3/9/2020
ChiCTR	2020/2/12	2/27/2020
ChiCTR	2020/2/12	2/28/2020

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ChiCTR	2020/2/12	3/1/2020
ChiCTR	2020/2/12	2/27/2020
ChiCTR	2020/2/12	2/14/2020
ChiCTR	2020/2/13	2/13/2020
ChiCTR	2020/2/13	2/13/2020
ChiCTR	2020/2/13	2/14/2020
ChiCTR	2020/2/13	2/13/2020
ChiCTR	2020/2/13	2/13/2020
ChiCTR	2020/2/13	2/18/2020
ChiCTR	2020/2/13	2/18/2020

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ChiCTR	2020/2/13	2/13/2020
ChiCTR	2020/2/13	2/16/2020
ChiCTR	2020/2/13	2/14/2020
ClinicalTrials.gov	2020/2/13	4/8/2021
ChiCTR	2020/2/14	12/30/2020
ChiCTR	2020/2/14	2/14/2020
ChiCTR	2020/2/14	2/14/2020
ChiCTR	2020/2/14	8/10/2020
ChiCTR	2020/2/14	3/15/2020
ClinicalTrials.gov	2020/2/14	4/14/2020

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ClinicalTrials.gov	2020/2/14	5/1/2020
ChiCTR	2020/2/15	3/15/2020
ChiCTR	2020/2/15	2/15/2020
ChiCTR	2020/2/15	2/24/2020
ChiCTR	2020/2/15	2/16/2020
ChiCTR	2020/2/15	2/18/2020
ChiCTR	2020/2/15	2/16/2020
ChiCTR	2020/2/15	3/22/2020
ChiCTR	2020/2/15	3/17/2020
ClinicalTrials.gov	2020/2/15	5/11/2020

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ClinicalTrials.gov	2020/2/15	1/3/2022
ChiCTR	2020/2/16	3/2/2020
ChiCTR	2020/2/16	3/1/2020
ChiCTR	2020/2/16	2/17/2020
ChiCTR	2020/2/16	2/17/2020
ChiCTR	2020/2/16	2/17/2020
ChiCTR	2020/2/17	2/17/2020
ChiCTR	2020/2/17	3/17/2020
ChiCTR	2020/2/17	7/22/2021
ChiCTR	2020/2/18	2/18/2020

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Report-902	ChiCTR	2020/2/18	3/17/2020
	ChiCTR	2020/2/18	2/18/2020
	ChiCTR	2020/2/18	2/18/2020
	ChiCTR	2020/2/18	2/18/2020
	ChiCTR	2020/2/18	3/16/2020
	ChiCTR	2020/2/18	2/19/2020
	ChiCTR	2020/2/18	3/9/2020
	ChiCTR	2020/2/18	2/19/2020
	ChiCTR	2020/2/19	6/17/2020
	ChiCTR	2020/2/19	2/24/2020

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ChiCTR	2020/2/19	2/19/2020
ChiCTR	2020/2/19	2/19/2020
ChiCTR	2020/2/19	2/28/2020
ChiCTR	2020/2/19	2/19/2020
ChiCTR	2020/2/19	2/20/2020
ClinicalTrials.gov	2020/2/19	7/7/2021
ChiCTR	2020/2/20	2/21/2020
ChiCTR	2020/2/20	3/5/2020
ChiCTR	2020/2/20	3/1/2020
ChiCTR	2020/2/20	3/24/2020

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ChiCTR	2020/2/21	3/16/2020
ChiCTR	2020/2/21	2/21/2020
ChiCTR	2020/2/21	2/21/2020
ChiCTR	2020/2/21	2/21/2020
ClinicalTrials.gov	2020/2/21	3/17/2020
ChiCTR	2020/2/22	2/22/2020
ChiCTR	2020/2/22	3/16/2020
ChiCTR	2020/2/22	2/22/2020
ChiCTR	2020/2/22	3/15/2020
ChiCTR	2020/2/22	2/22/2020

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ChiCTR	2020/2/22	2/23/2020
ChiCTR	2020/2/22	2/23/2020
ChiCTR	2020/2/23	2/23/2020
ChiCTR	2020/2/23	2/23/2020
ChiCTR	2020/2/23	2/23/2020
ChiCTR	2020/2/23	3/17/2020
ChiCTR	2020/2/24	3/14/2020
ChiCTR	2020/2/24	2/24/2020
ChiCTR	2020/2/24	3/17/2020
ChiCTR	2020/2/24	2/24/2020

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ChiCTR	2020/2/24	2/24/2020
ChiCTR	2020/2/24	2/24/2020
ChiCTR	2020/2/24	3/16/2020
ChiCTR	2020/2/24	3/17/2020
ClinicalTrials.gov	2020/2/24	8/19/2020
ClinicalTrials.gov	2020/2/24	3/18/2020
ChiCTR	2020/2/25	3/27/2020
ChiCTR	2020/2/25	3/17/2020
ChiCTR	2020/2/25	2/26/2020
ChiCTR	2020/2/26	3/17/2020

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ChiCTR	2020/2/26	2/26/2020
ChiCTR	2020/2/26	3/23/2020
ChiCTR	2020/2/26	2/26/2020
ChiCTR	2020/2/26	8/4/2020
ChiCTR	2020/2/26	2/26/2020
ChiCTR	2020/2/26	2/26/2020
ChiCTR	2020/2/26	2/27/2020
ChiCTR	2020/2/27	1/7/2022
ChiCTR	2020/2/28	2/29/2020
ChiCTR	2020/2/28	2/28/2020

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ChiCTR	2020/2/28	4/10/2020
ChiCTR	2020/2/28	3/14/2020
ClinicalTrials.gov	2020/2/28	7/9/2020
ChiCTR	2020/2/29	7/9/2020
ChiCTR	2020/3/1	4/19/2021
ChiCTR	2020/3/1	3/1/2020
ChiCTR	2020/3/1	3/13/2020
ChiCTR	2020/3/1	3/3/2020
ChiCTR	2020/3/1	3/20/2020
ChiCTR	2020/3/1	3/1/2020

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ChiCTR	2020/3/2	3/16/2020
ChiCTR	2020/3/2	3/2/2020
ChiCTR	2020/3/2	3/17/2020
ChiCTR	2020/3/2	3/8/2020
ChiCTR	2020/3/3	7/9/2020
ChiCTR	2020/3/3	3/13/2020
ChiCTR	2020/3/3	3/16/2020
ChiCTR	2020/3/3	3/3/2020
ChiCTR	2020/3/3	3/3/2020
ChiCTR	2020/3/3	3/14/2020

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ChiCTR	2020/3/3	3/3/2020
ClinicalTrials.gov	2020/3/4	3/16/2020
ChiCTR	2020/3/5	3/14/2020
ChiCTR	2020/3/5	3/5/2020
ChiCTR	2020/3/6	10/5/2020
ChiCTR	2020/3/6	3/6/2020
ChiCTR	2020/3/6	3/7/2020
ChiCTR	2020/3/6	3/16/2020
ChiCTR	2020/3/8	3/8/2020
ChiCTR	2020/3/8	3/15/2020

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Report-903	ChiCTR	2020/3/8	3/8/2020
	ClinicalTrials.gov	2020/3/9	4/10/2020
	ChiCTR	2020/3/9	3/9/2020
	ClinicalTrials.gov	2020/3/9	3/26/2020
	ChiCTR	2020/3/10	3/10/2020
	ChiCTR	2020/3/10	3/10/2020
	ChiCTR	2020/3/10	3/10/2020
	ChiCTR	2020/3/10	3/16/2020
	ChiCTR	2020/3/10	3/26/2020
	ChiCTR	2020/3/11	3/12/2020

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ChiCTR	2020/3/11	3/12/2020
ChiCTR	2020/3/13	4/12/2020
ChiCTR	2020/3/13	3/16/2020
ChiCTR	2020/3/14	3/14/2020
ChiCTR	2020/3/15	3/16/2020
ClinicalTrials.gov	2020/3/15	7/7/2021
ChiCTR	2020/3/16	3/16/2020
ChiCTR	2020/3/16	3/16/2020
ChiCTR	2020/3/17	3/23/2020
ChiCTR	2020/3/17	3/17/2020

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ChiCTR	2020/3/17	3/23/2020
ChiCTR	2020/3/17	3/18/2020
ChiCTR	2020/3/18	3/18/2020
ChiCTR	2020/3/18	3/23/2020
ChiCTR	2020/3/18	3/19/2020
ClinicalTrials.gov	2020/3/20	3/24/2020
ChiCTR	2020/3/20	3/23/2020
ChiCTR	2020/3/20	3/20/2020
ChiCTR	2020/3/22	3/22/2020
ChiCTR	2020/3/22	3/23/2020

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ChiCTR	2020/3/23	5/4/2020
ClinicalTrials.gov	2020/3/23	4/15/2020
ChiCTR	2020/3/24	3/24/2020
ClinicalTrials.gov	2020/3/25	4/7/2020
ClinicalTrials.gov	2020/3/25	11/17/2020
ClinicalTrials.gov	2020/3/25	5/1/2020
ChiCTR	2020/3/26	3/26/2020
ChiCTR	2020/3/27	3/27/2020
ClinicalTrials.gov	2020/3/28	3/10/2021
ChiCTR	2020/4/1	4/1/2020

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ChiCTR	2020/4/1	4/1/2020
ClinicalTrials.gov	2020/4/1	4/24/2020
ClinicalTrials.gov	2020/4/1	4/24/2020
ChiCTR	2020/4/3	4/3/2020
ChiCTR	2020/4/6	9/14/2020
ClinicalTrials.gov	2020/4/7	5/18/2020
ChiCTR	2020/4/8	4/8/2020
ChiCTR	2020/4/11	4/13/2020
ChiCTR	2020/4/11	4/11/2020
ChiCTR	2020/4/13	4/13/2020

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	ClinicalTrials.gov	2020/4/14	7/29/2021
	ChiCTR	2020/4/15	4/15/2020
Report-911	ChiCTR	2020/4/16	9/21/2020
	ChiCTR	2020/4/17	4/17/2020
	ChiCTR	2020/4/20	4/27/2020
	ClinicalTrials.gov	2020/4/20	11/24/2021
	ChiCTR	2020/4/21	8/31/2020
	ChiCTR	2020/4/23	4/23/2020
	ChiCTR	2020/4/23	4/23/2020
	ChiCTR	2020/4/24	4/24/2020

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ClinicalTrials.gov	2020/4/24	4/28/2020
ChiCTR	2020/4/25	4/26/2020
ChiCTR	2020/4/25	4/25/2020
ChiCTR	2020/4/26	7/9/2020
ChiCTR	2020/4/27	4/27/2020
ChiCTR	2020/4/29	2/1/2021
ChiCTR	2020/4/29	8/14/2020
ChiCTR	2020/4/29	4/29/2020
ChiCTR	2020/5/2	1/25/2021
ChiCTR	2020/5/4	5/10/2020

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ChiCTR	2020/5/8	5/8/2020
ChiCTR	2020/5/9	5/9/2020
ClinicalTrials.gov	2020/5/9	8/18/2021
ClinicalTrials.gov	2020/5/13	7/21/2020
ChiCTR	2020/5/15	5/15/2020
ChiCTR	2020/5/15	7/9/2020
ChiCTR	2020/5/19	5/19/2020
ChiCTR	2020/5/21	5/25/2020
ClinicalTrials.gov	2020/5/21	6/4/2020
ChiCTR	2020/5/25	6/1/2020

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	ChiCTR	2020/5/26	6/1/2020
	ChiCTR	2020/5/26	5/26/2020
	ChiCTR	2020/5/27	5/28/2020
Report-904	ChiCTR	2020/5/27	5/28/2020
	ClinicalTrials.gov	2020/6/6	6/22/2020
	ChiCTR	2020/6/11	6/11/2020
	ClinicalTrials.gov	2020/6/16	12/3/2020
	ISRCTN	2020/6/22	11/17/2020
	ChiCTR	2020/6/24	7/9/2020
	ClinicalTrials.gov	2020/7/8	12/3/2020

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	ClinicalTrials.gov	2020/7/13	7/14/2020
	ClinicalTrials.gov	2020/7/14	4/12/2021
	ClinicalTrials.gov	2020/7/14	4/12/2021
	ClinicalTrials.gov	2020/7/17	3/23/2021
Report-905	ChiCTR	2020/7/19	7/20/2020
	ChiCTR	2020/8/15	8/30/2020
	ClinicalTrials.gov	2020/8/16	1/27/2021
	ClinicalTrials.gov	2020/8/18	11/11/2021
	ClinicalTrials.gov	2020/8/22	2/26/2021
	ClinicalTrials.gov	2020/8/27	2/11/2021

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ChiCTR	2020/8/29	10/26/2020
ChiCTR	2020/9/1	9/8/2020
ChiCTR	2020/9/11	1/20/2021
ClinicalTrials.gov	2020/9/15	11/9/2020
ChiCTR	2020/9/18	12/14/2020
ChiCTR	2020/9/19	12/14/2020
ClinicalTrials.gov	2020/9/22	11/27/2020
ClinicalTrials.gov	2020/9/23	12/28/2021
ChiCTR	2020/9/30	9/30/2020
ChiCTR	2020/10/2	12/21/2020

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ClinicalTrials.gov	2020/10/19	10/22/2020
ChiCTR	2020/10/22	1/19/2021
ChiCTR	2020/10/24	1/27/2021
ChiCTR	2020/10/28	1/30/2021
ClinicalTrials.gov	2020/11/4	7/27/2021
ChiCTR	2020/11/6	2/11/2021
ClinicalTrials.gov	2020/11/12	2/26/2021
ClinicalTrials.gov	2020/11/17	2/15/2021
ClinicalTrials.gov	2020/11/17	1/8/2021
ClinicalTrials.gov	2020/11/19	3/30/2021

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ChiCTR	2020/11/22	11/22/2021
ChiCTR	2020/11/22	11/22/2020
ClinicalTrials.gov	2020/12/7	12/11/2020
ChiCTR	2020/12/11	4/1/2021
ClinicalTrials.gov	2020/12/17	1/27/2021
ChiCTR	2021/1/1	3/21/2021
ChiCTR	2021/1/1	3/21/2021
ChiCTR	2021/1/1	3/21/2021
ChiCTR	2021/1/8	3/26/2021
ChiCTR	2021/1/12	1/14/2022

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ChiCTR	2021/1/12	1/13/2022
ChiCTR	2021/1/13	1/14/2022
ChiCTR	2021/1/14	4/9/2021
ClinicalTrials.gov	2021/1/20	5/25/2021
ChiCTR	2021/1/29	5/11/2021
ChiCTR	2021/2/4	2/4/2021
ChiCTR	2021/2/5	5/22/2021
ClinicalTrials.gov	2021/3/4	1/25/2022
ChiCTR	2021/3/5	7/5/2021
ClinicalTrials.gov	2021/3/15	7/26/2021

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ClinicalTrials.gov	2021/3/19	5/25/2021
ChiCTR	2021/3/30	10/24/2021
ClinicalTrials.gov	2021/4/3	8/11/2021
ChiCTR	2021/4/7	11/8/2021
ChiCTR	2021/4/7	11/8/2021
ChiCTR	2021/4/7	11/8/2021
ClinicalTrials.gov	2021/4/8	8/2/2021
ClinicalTrials.gov	2021/4/24	5/7/2021
ClinicalTrials.gov	2021/4/27	5/13/2021
ChiCTR	2021/5/1	12/13/2021

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ClinicalTrials.gov	2021/5/7	9/24/2021
ChiCTR	2021/5/8	12/17/2021
ChiCTR	2021/5/11	12/19/2021
ChiCTR	2/4/2021	5/20/2021
ClinicalTrials.gov	5/24/2021	11/8/2021
ClinicalTrials.gov	5/30/2021	6/8/2021
ClinicalTrials.gov	5/10/2021	6/30/2021
ChiCTR	7/5/2021	7/5/2021
ChiCTR	6/27/2021	1/16/2022
ChiCTR	6/20/2021	1/14/2022

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ClinicalTrials.gov	7/6/2021	7/8/2021
ClinicalTrials.gov	6/30/2021	7/22/2021
ClinicalTrials.gov	7/5/2021	7/27/2021
ClinicalTrials.gov	7/8/2021	7/23/2021
ClinicalTrials.gov	7/26/2021	8/3/2021
ClinicalTrials.gov	8/2/2021	8/4/2021
ClinicalTrials.gov	8/2/2021	8/6/2021
ClinicalTrials.gov	8/10/2021	8/18/2021
ClinicalTrials.gov	8/10/2021	8/18/2021
ClinicalTrials.gov	5/10/2021	8/18/2021

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ClinicalTrials.gov	8/17/2021	8/20/2021
ChiCTR	8/16/2021	8/16/2021
ChiCTR	8/10/2021	8/10/2021
ChiCTR	8/8/2021	8/30/2021
ChiCTR	8/8/2021	8/30/2021
ChiCTR	8/5/2021	8/5/2021
ChiCTR	8/2/2021	8/2/2021
ChiCTR	8/2/2021	8/2/2021
ChiCTR	8/2/2021	8/2/2021
ChiCTR	7/31/2021	7/31/2021

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ChiCTR	7/21/2021	7/21/2021
ChiCTR	9/4/2021	9/4/2021
ClinicalTrials.gov	2/6/2020	9/8/2021
ClinicalTrials.gov	9/5/2021	9/9/2021
ChiCTR	3/24/2021	9/19/2021
ClinicalTrials.gov	9/13/2021	9/16/2021
ChiCTR	9/26/2021	9/26/2021
ClinicalTrials.gov	7/3/2021	9/29/2021
ClinicalTrials.gov	7/13/2021	9/24/2021
ClinicalTrials.gov	10/5/2021	10/15/2021

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ClinicalTrials.gov	6/13/2021	10/21/2021
ClinicalTrials.gov	10/14/2021	10/25/2021
ClinicalTrials.gov	10/12/2021	10/25/2021
ChiCTR	10/9/2021	10/10/2021
ChiCTR	9/30/2021	10/3/2021
ChiCTR	9/29/2021	10/3/2021
ChiCTR	11/6/2021	11/6/2021
ChiCTR	11/6/2021	11/6/2021
ChiCTR	9/30/2021	11/5/2021
ClinicalTrials.gov	11/1/2021	11/4/2021

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ChiCTR	11/24/2021	11/30/2021
ClinicalTrials.gov	11/2/2021	11/23/2021
ChiCTR	8/9/2021	12/7/2021
ChiCTR	4/20/2021	12/6/2021
ClinicalTrials.gov	11/8/2021	12/14/2021
ClinicalTrials.gov	12/3/2021	12/28/2021
ClinicalTrials.gov	12/20/2021	12/21/2021
ClinicalTrials.gov	12/20/2021	12/21/2021
ClinicalTrials.gov	12/24/2021	12/29/2021
ChiCTR	5/23/2021	1/2/2022

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ClinicalTrials.gov	12/13/2021	1/20/2022
ClinicalTrials.gov	1/12/2022	1/31/2022
ClinicalTrials.gov	1/12/2022	1/31/2022
ChiCTR	8/31/2021	11/4/2021
ChiCTR	9/5/2021	9/7/2021
ChiCTR	10/12/2021	10/12/2021
ChiCTR	11/27/2021	11/27/2021
ChiCTR	11/30/2021	11/30/2021
ChiCTR	12/13/2021	12/13/2021
ClinicalTrials.gov	1/21/2022	1/24/2022

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ClinicalTrials.gov	12/16/2021	1/19/2022
ClinicalTrials.gov	10/2/2021	2/1/2022
ClinicalTrials.gov	11/16/2021	1/3/2022
ClinicalTrials.gov	11/17/2021	1/11/2022
ChiCTR	2022/1/15	1/15/2022
ChiCTR	2022/1/19	1/19/2022

For peer review only

Scientific/Official Title

A randomized, controlled open-label trial to evaluate the efficacy and safety of lopinavir-ritonavir in hospitalized patients with 2019-nCoV infection

Glucocorticoid Therapy for Critically Ill Patients With Severe Acute Respiratory Infections Caused by COVID-19: a Prospective, Randomized Controlled Trial

Effects of Traditional Chinese Medicines (TCMs) on Patients With COVID-19 Infection: A Perspective, Open-labeled, Randomized, Controlled Trial

A study on treatment strategies of novel coronavirus pnueumonia patients

Comparative effectiveness and safety of ribavirin plus interferon-alpha, lopinavir/ritonavir plus interferon-alpha and ribavirin plus lopinavir/ritonavir plus interferon-alphain in patients with mild to moderate novel coronavirus pneumonia

Efficacy and Safety of Darunavir and Cobicistat for Treatment of COVID-19

Chinese Herbal medicine for Severe nevel coronavirus pneumonia (COVID-19): a Randomized Controlled Trial

Washed Microbiota Transplantation for Patients With 2019-nCoV Infection: a Randomized, Double-blind, Placebo-controlled Study

A Randomized, Open-label, Controlled Study of the Efficacy of Lopinavir Plus Ritonavir and Arbidol for Treating With Patients With Novel Coronavirus Infection

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A Phase 3 Randomized, Double-blind, Placebo-controlled Multicenter Study to Evaluate the Efficacy and Safety of Remdesivir in Hospitalized Adult Patients With Mild and Moderate COVID-19.

A Phase 3 Randomized, Double-blind, Placebo-controlled, Multicenter Study to Evaluate the Efficacy and Safety of Remdesivir in Hospitalized Adult Patients With Severe COVID-19.

Clinical study of type I macrophages therapy in the treatment of novel coronavirus pneumonia (COVID-19)

A randomized, open-label, blank-controlled trial for Lian-Hua Qing-Wen Capsule/Granule in the treatment of suspected novel coronavirus pneumonia (COVID-19)

A randomized, open-label, blank-controlled trial for Lian-Hua Qing-Wen Capsule /Granule in the treatment of novel coronavirus pneumonia (COVID-19)

Randomized controlled trial for traditional Chinese medicine in the prevention of novel coronavirus pneumonia (COVID-19) in high risk population

A randomized controlled trial of integrated TCM and Western Medicine in the treatment of severe novel coronavirus pneumonia (COVID-19)

Combination of traditional chinese medicne and western medicine in the treatment of common type novel coronavirus pneumonia (COVID-19)

Pulmonary rehabilitation to improve pulmonary function and quality of life in patients with novel coronavirus pneumonia (COVID-19) in rehabilitation period

The effect of shadowboxing for pulmonary function and quality of life in patients with novel coronavirus pneumonia (COVID-19) in rehabilitation period

For peer review only

A Randomized Controlled Trial for Integrated Traditional Chinese Medicine and Western Medicine in the Treatment of Common Type Novel Coronavirus Pneumonia (COVID-19)

Traditional Chinese Medicine for Pulmonary Fibrosis, Pulmonary Function and Quality of Life in Patients With Novel Coronavirus Pneumonia (COVID-19) in Convalescent Period: a Randomized Controlled Trial

Traditional Chinese Medicine, Psychological Intervention and Investigation of Mental Health for Patients With Novel Coronavirus Pneumonia (COVID-19) in Convalescent Period

An Open, Prospective/Retrospective, Randomized Controlled Cohort Study to Compare the Efficacy of Two Therapeutic Schemes(Abidol Hydrochloride,Abidol Hydrochloride Combined With Interferon Atomization)in the Treatment of 2019-nCoV Pneumonia.

An Open, Prospective/Retrospective, Randomized Controlled Cohort Study to Compare the Efficacy of Three Antiviral Drugs(Abidol Hydrochloride, Oseltamivir and Lopinavir/Ritonavir) in the Treatment of 2019-nCoV Pneumonia.

An Open, Prospective/Retrospective, Randomized Controlled Cohort Study to Compare the Efficacy of Different Hormone Doses in the Treatment of 2019-nCoV Severe Pneumonia

A randomized, open label, parallel controlled trial for evaluating the efficacy of recombinant cytokine gene-derived protein injection on clearing novel coronavirus in patients with novel coronavirus pneumonia (COVID-19)

Chinese medicine prevention and treatment program for novel coronavirus pneumonia (COVID-19)

A prospective randomized double-blind placebo-controlled study of traditional Chinese medicine staging regimen in the treatment of novel coronavirus pneumonia (COVID-19)

A randomized, open-label study to evaluate the efficacy and safety of Lopinavir-Ritonavir in patients with mild novel coronavirus pneumonia (COVID-19)

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A randomised, open, controlled trial for darunavir/cobicistat or Lopinavir/ritonavir combined with thymosin a1 in the treatment of novel coronavirus pneumonia (COVID-19)

A randomized controlled trial for the efficacy and safety of Baloxavir Marboxil, Favipiravir tablets in novel coronavirus pneumonia (COVID-19) patients who are still positive on virus detection under the current antiviral therapy

Randomized, open-label, controlled trial for evaluating of the efficacy and safety of Baloxavir Marboxil, Favipiravir, and Lopinavir-Ritonavir in the treatment of novel coronavirus pneumonia (COVID-19) patients

Recommendations of Integrated Traditional Chinese and Western Medicine for Novel Coronavirus Pneumonia (COVID-19)

Therapeutic effect of hydroxychloroquine on novel coronavirus pneumonia (COVID-19)

Safety and efficacy of umbilical cord blood mononuclear cells conditioned medium in the treatment of severe and critically novel coronavirus pneumonia (COVID-19): a randomized controlled trial

A Randomized,Open,Controlled Small Sample Clinical Study to Evaluate the Efficacy and Safety of ASC09/Ritonavir Compound Tablets and Ritonavir for 2019-nCoV Pneumonia

Vitamin C Infusion for the Treatment of Severe 2019-nCoV Infected Pneumonia: a Prospective Randomized Clinical Trial

Safety and efficacy of umbilical cord blood mononuclear cells in the treatment of severe and critically novel coronavirus pneumonia(COVID-19): a randomized controlled clinical trial

A multicenter, randomized, open, controlled clinical study to evaluate the efficacy and safety of recombinant cytokine gene-derived protein injection in combination with standard therapy in patients with novel coronavirus infection

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Severe novel coronavirus pneumonia (COVID-19) patients treated with ruxolitinib in combination with mesenchymal stem cells: a prospective, single blind, randomized controlled clinical trial

A prospective, randomized,open, multi-center clinical trial evaluating the efficiency and safety of Reduning injection in the treatment of novel coronavirus pneumonia (COVID-19)

Community based prevention and control for Chinese medicine in the treatment of novel coronavirus pneumonia (COVID-19) in the isolated population

Clinical study for community based prevention and control strategy of novel coronavirus pneumonia (COVID-19) in the isolate suspected and confirmed population

A Randomized, Open-Label, Multi-Centre Clinical Trial Evaluating and Comparing the Safety and Efficiency of ASC09/Ritonavir and Lopinavir/Ritonavir for Confirmed Cases of Novel Coronavirus Pneumonia (COVID-19)

A randomized, open-label, blank-controlled, multicenter trial for Shuang-Huang-Lian oral solution in the treatment of novel coronavirus pneumonia (COVID-19)

Efficacy and Safety of Hydroxychloroquine for Treatment of COVID-19

Clinical Study for Human Menstrual Blood-derived Stem Cells in the Treatment of Acute Novel Coronavirus Pneumonia (COVID-19)

A Multicenter, Randomized, Controlled trial for Recombinant Super-Compound Interferon (rSIFN-co) in the Treatment of Novel Coronavirus (2019-nCoV) Pneumonia

A randomized, open-label study to evaluate the efficacy and safety of low-dose corticosteroids in hospitalized patients with novel coronavirus pneumonia (COVID-19)

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Nasal high-fow preoxygenation assisted fibre-optic bronchoscope intubation in patients with critical novel coronavirus pneumonia (COVID-19): a randomized clinical trial

Efficacy of therapeutic effects of hydroxycholoroquine in novel coronavirus pneumonia (COVID-19) patients(randomized open-label control clinical trial)

Efficacy of Chloroquine and Lopinavir/ Ritonavir in mild/general novel coronavirus (CoVID-19) infections: a prospective, open-label, multicenter randomized controlled clinical study

A randomized, parallel controlled trial for the efficacy and safety of Sodium Aescinate Injection in the treatment of patients with pneumonia (COVID-19)

Effect evaluation and prognosis of Chinese medicine based on Novel Coronavirus Pneumonia (COVID-19)

A randomized, open, parallel-controlled clinical trial on the efficacy and safety of Jingyebaidu granules in treating novel coronavirus pneumonia (COVID-19)

Clinical study of nebulized Xiyanping injection in the treatment of novel coronavirus pneumonia (COVID-19)

Convalescent plasma for the treatment of severe and critical novel coronavirus pneumonia (COVID-19): a prospective randomized controlled trial

A multicenter, randomized, open label, controlled trial for the efficacy and safety of ASC09/ Ritonavir compound tablets and Lopinavir/ Ritonavir (Kaletra) and Arbidol tablets in the treatment of novel coronavirus pneumonia (COVID-19)

A randomized controlled study to evaluate the efficacy and safety of hydroxychloroquine for mild and moderate COVID-19 Infectious diseases

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Multicenter, randomized, open clinical study of Hydroxychloroquine Sulfate tablets in the treatment of common patients with novel coronavirus pneumonia (COVID-19)

A randomized, open-label, controlled trial for the efficacy and safety of hydroxychloroquine sulfate tablets in the treatment of patients with severe novel coronavirus pneumonia (COVID-19)

Study on Prevention and Treatment of Novel Coronavirus Pneumonia (COVID-19) With Traditional Chinese Medicine

A multicenter, randomized controlled trial for the efficacy and safety of tocilizumab in the treatment of new coronavirus pneumonia (COVID-19)

A randomized, open, controlled trial for diammonium glycyrrhizinate enteric-coated capsules combined with vitamin C tablets in the treatment of common novel coronavirus pneumonia (COVID-19) in the basic of clinical standard antiviral treatment to evaluate the safety and efficiency

Babaodan Capsule used for the adjuvant treatment of severe novel coronavirus pneumonia (COVID-19)

A randomized, open-label, blank-controlled, multicenter trial for Polyinosinic-Polycytidylic Acid Injection in the treatment of novel coronavirus pneumonia (COVID-19)

A multicenter, randomized, controlled trial for integrated chinese and western medicine in the treatment of novel coronavirus pneumonia (COVID-19) based on the 'Truncated Torsion' strategy

A multicenter, randomized, open, controlled trial for the efficacy and safety of Shen-Qi Fu-Zheng injection in the treatment of novel coronavirus pneumonia (COVID-19)

A multicenter, randomized, open and controlled trial for the efficacy and safety of Kang-Bing-Du granules in the treatment of novel coronavirus pneumonia (COVID-19)

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Traditional Chinese medicine cooperative therapy for patients with novel coronavirus pneumonia (COVID-19): a randomized controlled trial

Randomized controlled trial for TCM syndrome differentiation treatment impacting quality of life of post-discharge patients with novel coronavirus pneumonia (COVID-19)

Clinical study for the integration of traditional Chinese and western medicine in the treatment of novel coronavirus pneumonia (COVID-19)

Clinical Investigation of Natural Killer Cells Treatment in Pneumonia Patients Infected With 2019 Novel Coronavirus

A prospective, randomized, open-label, controlled clinical study to evaluate the preventive effect of hydroxychloroquine on close contacts after exposure to the Novel Coronavirus Pneumonia (COVID-19)

Optimization of treatment and diagnosis plan for critically ill patients

Clinical Trial for Tanreqing Capsules in the Treatment of Novel Coronavirus Pneumonia (COVID-19)

A randomized controlled trial for honeysuckle decoction in the treatment of patients with novel coronavirus (COVID-19) infection

A randomized, double-blind, parallel, controlled trial for comparison of phosphoric chloroquine combined with standard therapy and standard therapy in serious/critically ill patients with novel coronavirus pneumonia (COVID-19)

Clinical Study of Human Umbilical Cord Mesenchymal Stem Cells in the Treatment of Severe COVID-19

For peer review only

Evaluating the Efficacy and Safety of Bromhexine Hydrochloride Tablets Combined With Standard Treatment/ Standard Treatment in Patients With Suspected and Mild Novel Coronavirus Pneumonia (COVID-19)

A randomized, double-blind, parallel, controlled trial for comparison of phosphoric chloroquine combined with standard therapy and standard therapy in mild/common patients with novel coronavirus pneumon (COVID-19)ia

Application of Regulating Intestinal Flora in the Treatment of Severe Novel Coronavirus Pneumonia (COVID-19)

A multicenter, randomized controlled trial for the efficacy and safety of Alpha lipoic acid (iv) in the treatment of patients of severe novel coronavirus pneumonia (COVID-19)

A randomized, open-label, controlled clinical trial for azvudine in the treatment of novel coronavirus pneumonia (COVID-19)

A randomized, open and controlled clinical trial for traditional Chinese medicine in the treatment of novel coronavirus pneumonia (COVID-19)

The efficacy and safety of carrimycin treatment in patients with novel coronavirus pneumonia (COVID-19): a multicenter, randomized, open-label, controlled trial

Hydroxychloroquine treating novel coronavirus pneumonia (COVID-19): a randomized controlled, open label, multicenter trial

A multicenter, randomized, controlled trial for integrated Chinese and western medicine in the treatment of ordinary novel coronavirus pneumonia (COVID-19) based on the ' Internal and External Relieving -Truncated Torsion' strategy

Efficacy and Safety of Corticosteroids in COVID-19: A Prospective Randomized Controlled Trails

For peer review only

Efficacy and Safety of Fuzheng Huayu Tablets in Post-COVID-19 Patients With Pulmonary Inflammation and Fibrosis : A Multicenter Double-blind Randomized Controlled Trial

Evaluation the Efficacy and Safety of Hydroxychloroquine Sulfate in Comparison with Phosphate Chloroquine in Severe Patients with Novel Coronavirus Pneumonia (COVID-19): a Randomized, Open-Label, Parallel, Controlled Trial

Evaluation the Efficacy and Safety of Hydroxychloroquine Sulfate in Comparison with Phosphate Chloroquine in Mild and Commen Patients with Novel Coronavirus Pneumonia (COVID-19): a Randomized, Open-label, Parallel, Controlled Trial

A Single-blind, Randomized, Controlled Clinical Trial for Chloroquine Phosphate in the treatment of Novel Coronavirus Pneumonia 2019 (COVID-19)

A randomized controlled trial for Traditional Chinese Medicine in the treatment for Novel Coronavirus Pneumonia (COVID-19)

A Randomized Controlled Trial for Qingyi No. 4 Compound in the treatment of Convalescence Patients of Novel Coronavirus Pneumonia (COVID-19)

Efficacy and safety of honeysuckle oral liquid in the treatment of novel coronavirus pneumonia (COVID-19): a multicenter, randomized, controlled, open clinical trial

Chinese Medicine Promotes Rehabilitation Recommendations after 2019 Novel Coronavirus Infection (COVID-19)

A randomized controlled trial for the Efficacy of Ultra Short Wave Electrotherapy in the treatment of Novel Coronavirus Pneumonia (COVID-19)

A prospective, multicenter, open-label, randomized, parallel-controlled trial for probiotics to evaluate efficacy and safety in patients infected with 2019 novel coronavirus pneumonia (COVID-19)

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A randomized controlled trial for the efficacy of Dao-Yin in the prevention and controlling novel coronavirus pneumonia (COVID-19)

Clinical Study of Chloroquine Phosphate in the Treatment of Severe Novel Coronavirus Pneumonia (COVID-19)

A randomized controlled Trial for therapeutic efficacy of Recombinant Human Interferon alpha 1b Eye Drops in the treatment of elderly with novel coronavirus pneumonia (COVID-19)

Clinical trials of mesenchymal stem cells for the treatment of pneumonitis caused by novel coronavirus (COVID-19)

A randomized, open-label, controlled trial for the safety and efficiency of Kesuting syrup and Keqing capsule in the treatment of mild and moderate novel coronavirus pneumonia (COVID-19)

An open randomized controlled trial for Chloroquine phosphate and Hydroxychloroquine sulfate in the treatment of severe novel coronavirus pneumonia (COVID-19)

A multicenter, randomized, open-label, parallel controlled trial for the safety and efficacy of Integrated Chinese and Western Medicine in treatment of non-critical novel coronavirus pneumonia (COVID-19): pilot study

Liu-Zi-Jue Qigong and Acupressure Therapy for Pulmonary Function and Quality of Life in Patient with Severe novel coronavirus pneumonia (COVID-19): A Randomized Controlled Trial

The efficacy and safety of Triazavirin for 2019 novel coronary pneumonia (COVID-19): a multicenter, randomized, double blinded, placebo-controlled trial

Clinical study for novel NLRP Inflammasome inhibitor (Tranilast) in the treatment of novel coronavirus (COVID-19)

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Optimization Protocal of Integrated Traditional Chinese and Western Medicine Treatment for Novel Coronavirus Pneumonia (COVID-19)

A randomized controlled trial for the efficacy of ozonated autohemotherapy in the treatment of Novel Coronavirus Pneumonia (COVID-19)

Multicenter randomized controlled trial for rhG-CSF in the treatment of novel coronavirus pneumonia (COVID-19)

A randomized, double-blind, parallel-controlled, trial to evaluate the efficacy and safety of anti-SARS-CoV-2 virus inactivated plasma in the treatment of severe novel coronavirus pneumonia patients (COVID-19)

Efficacy evaluation of inhalation of mycobacterium vaccae injection for treating Novel coronavirus pneumonia (COVID-19) and the mechanism study of prevention and treatment of respiratory virus infection

Adaptive, Randomized, Single-blind, Three-arm Parallel Controlled Clinical Trial of Yinhu Qingwen Decoction in the Treatment of Mild / Common CoVID-19

A randomized, open-label, controlled trial for the efficacy and safety of Farpiravir Tablets in the treatment of patients with novel coronavirus pneumonia (COVID-19)

Effect of early pulmonary training on lung function and quality of life for novel coronavirus pneumonia (COVID-19) patients

A parallel, randomized controlled clinical trial for the efficacy and safety of Pediatric Huatanzhike granules (containing ipecacuanha tincture) in the treatment of mild and moderate novel coronavirus pneumonia (COVID-19)

Clinical comparative study of PD-1 mAb in the treatment of severe and critical patients with novel coronavirus disease (COVID-19)

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A randomized, double-blind, parallel, controlled trial for comparison of phosphoric chloroquine combined with standard therapy and standard therapy in mild/common patients with novel coronavirus pneumonia (COVID-19)

A study for the intervention of Xiangxue antiviral oral solution and Wu-Zhi-Fang-Guan-Fang on close contacts of novel coronavirus pneumonia (COVID-19)

Traditional Chinese Medicine in the treatment of novel coronavirus pneumonia (COVID-19): a multicentre, randomized controlled trial

Shen-Fu injection in the treatment of severe novel coronavirus pneumonia (COVID-19): a multicenter, randomized, open-label, controlled trial

A Randomized, Open Label, Controlled Clinical Study to Evaluate the Recombinant Human Angiotensin-converting Enzyme 2 (rhACE2) in Adult Patients With COVID-19

An open randomized controlled trial for Chloroquine phosphate and Hydroxychloroquine sulfate in the treatment of mild and common novel coronavirus pneumonia (COVID-19)

Multicenter study for the treatment of Dipyridamole with novel coronavirus pneumonia (COVID-19)

A multicenter, randomized, double-blind, controlled clinical trial for leflunomide in the treatment of novel coronavirus pneumonia (COVID-19)

Randomized controlled trial for the efficacy of dihydroartemisinine piperaquine in the treatment of mild/common novel coronavirus pneumonia (COVID-19)

A multicenter study for efficacy of intelligent psychosomatic adjustment system intervention in the treatment of novel coronavirus pneumonia (COVID-19) patients with mild to moderate anxiety and depression

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Umbilical cord Wharton's Jelly derived mesenchymal stem cells in the treatment of severe novel coronavirus pneumonia (COVID-19)

A randomized, open-label, controlled trial for the efficacy and safety of Adalimumab Injection in the treatment of patients with severe novel coronavirus pneumonia (COVID-19)

A multicenter randomized controlled trial for ozone autohemotherapy in the treatment of novel coronavirus pneumonia (COVID-19)

Safety and effectiveness of human umbilical cord mesenchymal stem cells in the treatment of acute respiratory distress syndrome of severe novel coronavirus pneumonia (COVID-19)

A multicenter, randomized, open, parallel controlled trial for the evaluation of the effectiveness and safety of Xiyanping injection in the treatment of common type novel coronavirus pneumonia (COVID-19)

A clinical study for "Huo-Shen" particles in the treatment of novel coronavirus pneumonia (COVID-19)

A randomized controlled trial for high-dose Vitamin C in the treatment of severe and critical novel coronavirus pneumonia (COVID-19) patients

Clinical Trial for Human Mesenchymal Stem Cells in the Treatment of Severe Novel Coronavirus Pneumonia (COVID-19)

Randomized, parallel control, open trial for Qing-Wen Bai-Du-Yin combined with antiviral therapy in the treatment of novel coronavirus pneumonia (COVID-19)

Clinical Trial for Recombinant Human Interleukin-2 in the Treatment of Novel Coronavirus Pneumonia (COVID-19)

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Key techniques of umbilical cord mesenchymal stem cells for the treatment of novel coronavirus pneumonia (COVID-19) and clinical application demonstration

Experimental study of novel coronavirus pneumonia rehabilitation plasma therapy severe novel coronavirus pneumonia (COVID-19)

Efficacy and safety of Lopinavir and Ritonavir in the treatment of novel coronavirus pneumonia (COVID-19)

Clinical Study on Syndrome Differentiation of TCM in Treating Severe and Critical novel coronavirus pneumonia (COVID-19)

A Phase II, Multicenter, Randomized, Double-blind, Placebo-controlled Trial to Evaluate the Efficacy and Safety of Human Umbilical Cord-derived Mesenchymal Stem Cells in the Treatment of Severe COVID-19 Patients

Human Umbilical Cord Mesenchymal Stem Cells Treatment for Pneumonia Patients Infected by 2019 Novel Coronavirus

Effect of pulmonary rehabilitation on pulmonary function, exercise capacity, activity of daily living and quality of life in survivors due to severe or critial novel coronavirus pneumonia (COVID-19)

Study for the efficacy of Kangguan No. 1-3 prescription in the treatment of novel coronavirus pneumonia (COVID19)

Study of Pinavir / Ritonavir Tablets (Trade Name: Kelizhi) Combined with Xiyanping Injection for Novel Coronavirus Pneumonia (COVID-19)

Clinical study of mesenchymal stem cells in treating severe novel coronavirus pneumonia (COVID-19)

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Clinical Reseach of Acupuncture in the treatment of Novel Coronavirus Pneumonia (COVID-19)

the Efficacy and Safety of Favipiravir for novel coronavirus–infected pneumonia: A multicenter, randomized, open, positive, parallel-controlled clinical study

Efficacy and Safety of Jing-Yin Granule in the treatment of novel coronavirus pneumonia (COVID-19) wind-heat syndrome

Evaluation Danoprevir sodium tablets combined with ritonavir in the treatment of novel coronavirus infection (COVID-19): a randomized, open and controlled trial

Clinical study on novel coronavirus pneumonia (COVID-19) patients in Tibetan Plateau promoted by individualized nutritional assessment and supportive treatment

A study for the key technology of mesenchymal stem cells exosomes atomization in the treatment of novel coronavirus pneumonia (COVID-19)

Clinical research program of continuous renal replacement therapy with adsorption filter for the treatment of the novel coronavirus pneumonia (COVID-19)

Study on Prevention and Treatment of Novel Coronavirus Pneumonia (COVID-19) With Traditional Chinese Medicine

A Randomized, Open-label Study to Evaluate the Efficacy and Safety of Pirfenidone in Patients With Severe and Critical Novel Coronavirus Infection

A prospective randomized controlled trial for a Self-made Gastroscope isolation mask for preventing and controlling the novel coronavirus pneumonia (COVID-19) Epidemic period

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Clinical application of inhaled acetylcysteine solution in the treatment of novel coronavirus pneumonia (COVID-19)

Clinical trial for umbilical cord blood CIK and NK cells in the treatment of mild and general patients infected with novel coronavirus pneumonia (COVID-19)

A Phase IIb Randomized Placebo-Controlled Study to Examine the Efficacy and Safety of DAS181 for the Treatment of Severe Influenza Infection

A randomized, open-label, controlled and single-center trial to evaluate the efficacy and safety of anti-SARS-CoV-2 inactivated convalescent plasma in the treatment of novel coronavirus pneumonia (COVID-19) patient

Efficacy and safety of Xue-Bi-Jing injection in the treatment of severe cases of novel coronavirus pneumonia (COVID-19)

A randomized, double-blind, placebo-controlled trial for evaluation of the efficacy and safety of bismuth potassium citrate capsules in the treatment of patients with novel coronavirus pneumonia (COVID-19)

Efficacy and safety of chloroquine phosphate inhalation combined with standard therapy in the treatment of novel coronavirus pneumonia (COVID-19)

Application of rehabilitation lung exercise eight-segment exercise in patients with novel coronavirus pneumonia (COVID-19)

A Clinical Trial Study for the Influence of TCM Psychotherapy on Negative Emotion of Patients with Novel Coronavirus Pneumonia (COVID-19) Based on Network Platform

Application of rehabilitation and Lung eight-segment exercise in front-line nurses in the prevention of novel coronavirus pneumonia (COVID-19) epidemic

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Influence of nasal high-fow preoxygenation on video laryngoscope for emergency intubation in patients with critical novel coronavirus pneumonia (COVID-19)

A Randomized Controlled Trial for the Influence of TCM Psychotherapy on Negative Emotion of Patients with Novel Coronavirus Pneumonia (COVID-19) Based on Network Platform

A randomized parallel controlled trial for LIUSHENWAN in Treatment of Novel Coronavirus Pneumonia (COVID-19)

Efficacy and safety of lipoic acid injection in reducing the risk of progression in common patients with novel coronavirus pneumonia (COVID-19)

Study for meditation assists for the rehabilitation of patients with novel coronaviruse pneumonia (COVID-19)

Positively Controlled Clinical Study of Compound Yuxingcao Mixture in the Treatment of the Novel Coronavirus Pneumonia (COVID-19)

Study for the Effectiveness and Safety of Yi-Qi Hua-shi Jie-Du-Fang in the Treatment of the Novel Coronavirus Pneumonia (COVID-19)

Randomized, open, blank controlled trial for the efficacy and safety of recombinant human interferon alpha 1beta in the treatment of Wuhan patients with novel coronavirus pneumonia (COVID-19)

The clinical value of corticosteroid therapy timing in the treatment of novel coronavirus pneumonia (COVID-19): a prospective randomized controlled trial

A multicentre, randomized, controlled trial for the Ba-Duan-Jin in the adjunctive treatment of patients with common type novel coronavirus pneumonia (COVID-19)

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HUMSCs and Exosomes Treating Patients with Lung Injury following Novel Coronavirus Pneumonia (COVID-19)

Clinical Study of Tetrandrine Tablets Adjuvant Treatment With COVID-19

Clinical Study of NK Cells in the Treatment of Novel Coronavirus Pneumonia (COVID-19)

Zedoary Turmeric Oil for Injection in the treatment of Novel Coronavirus Pneumonia (COVID-19): a randomized, open, controlled trial

Efficacy and safety of Ma-Xing-Gan-Shi decoction in the treatment of novel coronavirus pneumonia (COVID-19)

Multi-Center Clinical Study on the Treatment of Patients with Novel Coronavirus Pneumonia (COVID-19) by Ebastine

Research for the mechanism of improvement of novel coronavirus pneumonia (COVID-19) patients' pulmonary exudation by continuous renal replacement therapy

Efficacy and safety of honeysuckle oral liquid in the treatment of novel coronavirus pneumonia (COVID-19): a multicenter, randomized, controlled, open clinical trial

Clinical Prediction and Intervention of Pulmonary Function Impairment in Patients with Novel Coronavirus Pneumonia (COVID-19)

Tozumab combined with adamumab(Qletli) in the treatment of severe and critically ill patients with novel coronavirus pneumonia (COVID-19):A prospective, single-center, randomized, parallel controlled trial

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Study on the application of convalescent plasma therapy in severe COVID-19

Favipiravir Combined With Tocilizumab in the Treatment of Corona Virus Disease 2019-A Multicenter, Randomized and Controlled Clinical Trial Study

An anaesthesia procedure and extubation strategy for reducing patient agitation and cough after extubation that can be used to prevent the spread of SARS-CoV-2 and other infectious viruses in the operating Room

The Efficacy and Safety of Bevacizumab in Severe or Critical Patients With COVID-19--a Multicenter Randomized Controlled Clinical Trial

An evaluative clinical study: efficacy and safety of of Prolongin (Enoxaparin Sodium Injection) in treatment of hospitalized adult patients with common novel coronavirus pneumonia (COVID-19)

A randomized, parallel controlled open-label trial to evaluate the efficacy and safety of Prolongin (Enoxaparin Sodium Injection) in adult hospitalized patients with novel coronavirus pneumonia (COVID-19)

Convalescent plasma for the treatment of common COVID-19: a prospective randomized controlled trial

A randomized, blinded, controlled, multicenter clinical trial to evaluate the efficacy and safety of Ixekizumab combined with conventional antiviral drugs in patients with novel coronavirus pneumonia (COVID-19)

Observation Of Clinical Efficacy And Safety Of Bufonis Venenum Injection In The Treatment Of Severe Novel Coronavirus Pneumonia (COVID-19)

Clinical Study of Chloroquine Phosphate in the Treatment of 2019-nCoV Infection

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Prognosis Investigation and Intervention Study on Patients with novel coronavirus pneumonia (COVID-19) in recovery period Based on Community Health Management

A clinical study for effectiveness and safety evaluation for recombinant chimeric COVID-19 epitope DC vaccine in the treatment of novel coronavirus pneumonia

Study for the therapeutic effect and mechanism of traditional Chinese medicine in the treatment of novel coronavirus pneumonia (COVID-19)

A randomized, open-label, controlled trial for the efficacy and safety of Ulinastatin Injection in the treatment of patients with severe novel coronavirus pneumonia (COVID-19)

Exocarpium Citri Grandis Relieves Symptoms of Novel Coronavirus Pneunomia (COVID-19): a Randomized Controlled Clinical Trial

An Adaptive, Randomized, Double-blind, Parallel-controlled Clinical Trial of Yinhu Qingwen Granula for the Treatment of Severe CoVID-19

Efficacy and Safety of Pirfenidone in the Treatment of Severe Post-Novel Coronavirus Pneumonia (COVID-19)) Fibrosis:a prospective exploratory experimental medical study

Evaluation of the effect of taking Newgen beta-gluten probiotic composite powder to nutrition intervention of patients with novel coronavirus pneumonia (COVID-19)

Evaluation of the effect of taking tricholoma matsutake, cannabis sativa capsule and dendrobium candidum to nutrition intervention of patients with novel coronavirus pneumonia (COVID-19) during convalescence.

Prospective, open-label, controlled, multicenter cohort study of long-acting interferon plus ribavirin in patients with COVID-19 infection

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The treatment and diagnosis plan of integrated traditional Chinese and Western medicine for novel coronavirus pneumonia (COVID-19)

A randomized, double-blind, parallel-controlled trial to evaluate the efficacy and safety of anti-SARS-CoV-2 virus inactivated plasma in the treatment of severe novel coronavirus pneumonia (COVID-19)

Effectiveness of "Liu-Zi-Jue" combined with respiratory muscle training for respiratory function in COVID-19 patients: a demonstration study for the key techniques

Gu-Shen Ding-Chuan-Wan used in the recovery phase of 2019-nCoV pneumonia(FeiPiQiXuZheng): a randomized, open-label, controlled clinical trial

An open, multi-center, control, exploratory clinical study of human NK cells and UC-MSCs transplantation for severe novel coronavirus pneumonia

Clinical Trial of Favipiravir Tablets Combine With Chloroquine Phosphate in the Treatment of Novel Coronavirus Pneumonia

Efficacy and safety of Chinese Herb Medicine Hua-Shi Bai-Du granules in patients with novel coronavirus pneumonia (COVID-19) in Wuhan, China: a prospective, randomized, controlled, open-label trial

Effect of Auricular point pressing on insomnia of 2019 new coronavirus pneumonia patients: a randomized controlled trial.

Psychological Support for Diagnosed Patients with novel coronavirus pneumonia (COVID-19)

Prospective exploratory clinical research on the efficacy and safety of pirfenidone capsules in the treatment of severe novel coronavirus pneumonia (COVID-19)

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Effectiveness and safety of hydroxychloroquine sulfate in the preventive treatment of novel coronavirus pneumonia (COVID-19)

Safety and Efficacy of Intravenous Infusion of Bone Marrow-Derived Mesenchymal Stem Cells in Severe Patients With Coronavirus Disease 2019 (COVID-19): A Phase 1/2 Randomized Controlled Trial

A multicenter, single-blind, randomized controlled clinical trial for chloroquine phosphate in the treatment of 2019 novel coronavirus-infected pneumonia

A Multi-center, Randomized, Parallel-Controlled Clinical Trial of the Application of A Hydrogen-Oxygen Generator With Nebulizer in the Improvement of Symptoms in Patients Infected With COVID-19

A Phase I/II Study of Universal Off-the-shelf NKG2D-ACE2 CAR-NK Cells Secreting IL15 Superagonist and GM-CSF-neutralizing scFv for Therapy of COVID-19

Safety and Effectiveness of Mesenchymal Stem Cells in the Treatment of Pneumonia of Coronavirus Disease 2019

Metabolic abnormalities and rearrangement reversal treatment of patient with novel coronavirus pneumonia (COVID-19)

The protective effect of sleep psychology and music therapy in patients with new coronary pneumonia (mild and moderate type)

Safety and Efficacy Study of Allogeneic Human Dental Pulp Mesenchymal Stem Cells to Treat Severe Pneumonia of COVID-19:a Single-center, Prospective, Randomised Clinical Trial

Efficacy and safety of Nintedanib esilate soft capsules in the treatment of pulmonary fibrosis in healed moderate to severe patients of novel coronavirus pneumonia (COVID-19): a single-center, randomized, placebo-controlled trial

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Clinical study for prevention and treatment of digestive tract lesions caused by novel coronavirus pneumonia (COVID-19)

The Mechanism, Clinical Outcome and Therapeutic Intervention of Corona Virus Disease 2019 Patients Whose Nucleic Acids Changed From Negative to Positive

Efficacy and Safety of Anluohuaxian in the Treatment of Rehabilitation Patients With Corona Virus Disease 2019-A Multicenter, Open, Randomized Controlled Study

Application of non-invasive multichannel cardiopulmonary monitoring cloud management platform in patients with Novel Coronavirus Pneumonia (COVID-19)

Development and application of TCM body regulating protection scheme for the convalescent population of novel coronavirus pneumonia (COVID-19)

A Randomized, Double-blind, Placebo-controlled Phase II Clinical Trial to Evaluate the Safety and Immunogenicity of the Recombinant Novel Coronavirus Vaccine (Adenovirus Vector) in Healthy Adults Aged Above 18 Years

Clinical study for natural killer (NK) cells from umbilical cord blood in the treatment of viral pneumonia include novel coronavirus pneumonia (COVID-19)

Evaluation of the safety and immunogenicity of inactivated Novel Coronavirus Pneumonia (COVID-19) vaccine (Vero cells) in healthy population aged 6 years and above: a randomized, double-blind, placebo parallel-controlled phase I/II clinical trial

A clinical study for the effect of remote monitoring exercise rehabilitation on the discharged patients with novel coronavirus pneumonia (COVID-19)

Clinical Research on Treatment of Novel Coronavirus Pneumonia with "Guangdong Pneumonia NO.1"

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A Randomized, Double-Blinded, Placebo-Controlled, Phase I/II Clinical Trial, to Evaluate the Safety and Immunogenicity of the SARS-CoV-2 Inactivated Vaccine in Healthy Adults Aged 18~59 Years

Efficacy of Chinese Herbal Tea in the Prevention of Novel Coronavirus Pneumonia (COVID-19) for Returning Workers in Wuhan: a Randomized Controlled Trial

Study on the prevention of COVID-19 with Chinese medicine regulating constitution

Effecacy and Safety of Hyperbaric Oxygen Therapy to patients with novel coronavirus pneumonia (COVID-19)

Efficacy and Safety of ulinastatin in the Treatment of novel coronavirus pneumonia (COVID-19)

Moxibustion Plus Cupping in Convalescent Patients With COVID-19: A Randomized Clinical Trial

A multicenter, randomized, double-blind, parallel-controlled trial for Qi-Mai-Fei-Luo-Ping Mixture in the improvement of lung function of novel coronavirus pneumonia (COVID-19) in the convalescent period

A multicenter randomized, double-blind, placebo-controlled trial for Sheng-Mai-Yin for improvement of the pulmonary heart function related symptoms of convalescence patients of new coronavirus pneumonia

A randomized, double-blind, placebo-controlled, multicentre clinical study on the improvement of novel coronavirus pneumonia (COVID-19) convalescent sleep mood disorder with shumian capsule

A multicenter, randomized, open, controlled trial for the efficacy and safety of oral kolimycin in the treatment of patients with new coronavirus pneumonia (CoVID-19)

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Protective Effect of Aspirin on COVID-19 Patients

Study for efficacy and safety of Jie-Xing-Jun-Zi granules in the Treatment of convalescent patients of novel coronavious pneumonia (COVID-19)

A randomized clinical trial for the efficacy and safety of Aliskiren and Nifedipine in novel coronavirus pneumonia (COVID-19) patients with hypertension

Rqesearch on the Application of Traditional Chinese Medicine Traditional Techniques to Prevent and Control New Coronary Pneumonia

Xiaoyao capsule improves COVID-19 convalescence sleep mood disorder in a randomized, double-blind, placebo-controlled, multicenter clinical study

Application of pulmonary rehabilitation exercise in the treatment of novel coronavirus pneumonia (COVID-19) patients during convalescence period.

Evaluation of the safety and immunogenicity of inactivated novel coronavirus (2019-CoV) vaccine (Vero cells) in healthy population aged 3 years and above: a randomized, double-blind, placebo parallel-controlled phase I/II clinical trial

Study for using sulfate in the prevention and control of novel coronavirus pneumonia (COVID-19) in high and low prevalence communities

A randomized, double-blind, controlled trial for Bufei Huoxue Capsule in the treatment of convalescent COVID-19 patients

A randomized, double-blind, placebo, parallel controlled trial for Pien-Tze-Huang Capsules in treating convalescent novel coronavirus pneumonia (COVID-19) patient with SARS-CoV-2 positive.

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Clinical trial for the washed microbiota transplantation in the treatment of novel coronavirus pneumonia (COVID-19) patients suspected with gut microbiota dysbiosis

A randomized,double blinded, parallel controlled clinical trial for azvudine in the treatment of novel coronavirus pneumonia (COVID-19)

A Randomized, Double-blind, Placebo-controlled Clinical Trial, to Evaluate Safety and Immunogenicity of Inactivated SARS-CoV-2 Vaccine (Vero Cell), in Healthy Elderly Aged 60 Years and Above

The Effect of Berberine on Intestinal Function and Inflammatory Mediators in Severe Patients With Covid-19

A randomized controlled trial for the efficacy and safety of artemisinin-pipequine tablets in the treatment of the mild and common type novel coronavirus pneumonia (COVID-19) patients whose nCoV Nucleic acid did not turn negative after treated by hydroxychloroquine and Abidor

A multicenter, randomized, double-blind, placebo-controlled trial for Lu-Dang-Shen oral liquid in the treatment of reduced digestion function of convalescent new coronavirus pneumonia (COVID-19) patient

A randomized controlled trial for determination of the clinical effect of Internet-assisted pulmonary rehabilitation on patients with novel coronavirus pneumonia (COVID-19)

A randomized, open-label, blank-controlled, multicenter trial for Shuang-Huang-Lian oral solution in the treatment of novel coronavirus pneumonia (COVID-19)

A Randomized, Double-blind, Placebo-controlled, Phase Ia/IIa Trial of an Inactivated SARS-CoV-2 Vaccine in Healthy People Aged 18 to 59 Years

Usefulness of abdominal massage and ST-36 (Zusanli) acupressure in improving appetite of novel coronavirus pneumonia (COVID-19) patients treated in makeshift hospital

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Study for the improvement of the quality of life of severe patients of novel coronavirus pneumonia (COVID-19) by the integrated Chinese medicine and Western medicine therapy

A Randomized, Double-blind, Placebo-controlled trial for determination of the Efficacy and Safety of Baicalein in Hospitalized Adult Mild and Moderate Patients of novel coronavirus pneumonia (COVID-19)

Rqesearch on the Application of Traditional Chinese Medicine Traditional Techniques to Prevent and Control New Coronary Pneumonia

Study for Traditional Chinese Medicine in the treatment of novel coronavirus pneumonia (COVID-19)

A Randomized, Double-blind, Placebo-controlled, Phase I Clinical Study to Evaluate the Tolerability, Safety, Pharmacokinetic Profile and Immunogenicity of JS016 (Anti-SARS-CoV-2 Monoclonal Antibody) Injection in Chinese Healthy Subjects After Intravenous Infusion of Single Dose

Evaluating the Efficacy and Safety of Bromhexine Hydrochloride Tablets in Treating Pediatric novel coronavirus pneumonia (COVID-19)

A Multi-center, Double-blind, Randomized, Placebo Parallel Controlled, Safety and Tolerability Phase I Clinical Trial of Recombinant Novel Coronavirus Vaccine (CHO Cells) in Healthy People Between 18 and 59 Years of Age

The effect of cognitive behavioural therapy on depression, anxiety and stress in patients with COVID-19: a randomized controlled trial

A Phase I clinical trial to evaluate the safety, tolerance and preliminary immunogenicity of different doses of a SARS-CoV-2 mRNA vaccine in population aged 18-59 years and 60 years and above

A Randomized, Blinded, Placebo-controlled Trial to Evaluate the Immunogenicity and Safety of a Recombinant New Coronavirus Vaccine (CHO Cell) With Different Doses and Different Immunization Procedures in Healthy People Aged 18 to 59 Years

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A Randomized, Double-blind, Placebo-controlled, Phase Ib/IIb Trial of an Inac

tivated SARS-CoV-2 Vaccine in Healthy People Aged ⩾60 Years

A Phase 1 Randomized, Single-blind, Placebo-controlled, Single Ascending Dose Escalation Study of the Safety, Tolerability, and Pharmacokinetics of Human Monoclonal Antibody, BRII-196 Administered Intravenously to Healthy Adult Volunteers

A Phase 1 Randomized, Single-blind, Placebo-controlled, Single Ascending Dose Escalation Study of the Safety, Tolerability, and Pharmacokinetics of Human Monoclonal Antibody, BRII-198 Administered Intravenously to Healthy Adult Volunteers

A Randomized, Double-blinded, Placebo-controlled, Single Ascending Dose, Phase I Study to Evaluate the Tolerability, Safety, Pharmacokinetics of SCTA01 in Healthy Subjects

Sancai granule improves lung and kidney function for patient with COVID-19 pneumonia in the recovery period: a randomized, parallel controlled trial

Clinical study of safety, tolerance, and pharmacokinetics of nebulizer (INF-K + anti-inflammatory factor TFF2) in healthy volunteers and exploration of PK/PD fitting modeling in clinical trials of Novel Coronavirus Pneumonia (COVID-19) patients

A Randomized, Double-blind, Placebo-controlled Phase I Clinical Trial to Evaluate the Safety and Tolerability of Recombinant New Coronavirus Vaccines (CHO Cells) in Healthy People Aged 60 Years and Above

Safety and Immunogenicity of SARS-CoV-2 mRNA Vaccine (BNT162b1) in Chinese Healthy Subjects: A Phase I, Randomized, Placebo-controlled, Observer-blind Study

A Phase I Clinical Study to Evaluate the Safety, Tolerability, Pharmacokinetic Characteristics and Immunogenicity of a Single Dose of MW33 Injection in Healthy Subjects

Safety, Tolerability and Immunogenicity of a Recombinant SARS-CoV-2 Vaccine (Sf9 Cell) in Chinese Healthy Population Aged 18 Years and Older: A Phase I, Single-center, Randomized, Placebo-controlled, Double-blind Study

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Longitudinal research project of Wuhan novel coronavirus pneumonia (COVID-19) patients and important rehabilitation factors

A Phase I Clinical Trial of Influenza virus Vector COVID-19 Vaccine for intranasal Spray (DelNS1-2019-nCoV-RBD-OPT1)

A Single-center, Randomized, Double-blind, Placebo-controlled, Dose Ranging Phase I Study to Evaluate the Safety, Tolerability, and Immunogenicity of INO-4800, Administered Intradermally Followed by Electroporation in Healthy Volunteers

A Randomized, Double-Blinded, Placebo-Controlled, Phase I/II Clinical Trial, to Evaluate the Safety and Immunogenicity of the SARS-CoV-2 Inactivated Vaccine (Vero Cell) in Healthy Population Aged 3-17 Years

Study on Screening and prevention of osteonecrosis of the femoral head during novel coronavirus pneumonia (COVID-19) recovery

Study on TCM intervention programs for novel coronavirus pneumonia (COVID-19) Convalescence and Sequelae based on "COVID-19 case Registration System"

A Randomized, Double-blind, Placebo -Controlled Phase IIb Clinical Trial to Evaluate the Safety and Immunogenicity of Ad5-nCoV in Person 6 Years of Age and Older and Those Who Have Previously Been Vaccinated With Ad5-EBOV

A Randomized, Double-blind, Placebo-controlled, Phase Ib/II Clinical Study to Evaluate the Preliminary Efficacy, Safety, Pharmacokinetic Profiles and Immunogenicity of JS016 in Participants With Mild and Moderate COVID-19 or of SARS-CoV-2 Asymptomatic Infection

Study on traditional Chinese Medicine intervention Program of COVID-19 's convalescence and sequelae based on COVID-19 case Registration system

Evaluation of the safety and immunogenicity of inactivated SARS-CoV-2 Vaccine(Vero Cells) in healthy population aged 18 years and above: a randomized, double-blind, placebo parallel-controlled phase I clinical trial

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A Clinical Trial to Evaluate the Efficacy and Safety of Hydrogen-oxygen Mixed Gas Inhalation in Convalescent Severe/Critically Ill Patients With Novel Coronavirus Pneumonia (COVID-19)

A Phase Ib clinical trial to evaluate the safety, tolerance and preliminary immunogenicity of different doses of a SARS-CoV-2 mRNA vaccine in population aged 18-59 years

The Effects of Narrative Exposure Therapy on COVID-19 Patients with Post-Traumatic Stress Disorder: a Randomized Controlled Trial

Evaluation of the safety and immunogenicity of inactivated SARS-CoV-2 Vaccine (Vero Cells) in healthy population aged 18 years and above: a randomized, double-blind, placebo parallel-controlled phase II clinical trial

A Randomized, Double-Blinded Clinical Trial, to Evaluate the Non-inferiority of the Commercial Scale Inactivated SARS-CoV-2 Vaccine Against That of the Pilot Scale Among Adults Aged 26-45 Years, and the Open-labelled, Bridging Non-inferiority of the Vaccine Induced Immunogenicity in Elderly Against That in Adults

A Phase II Clinical Trial of Influenza virus Vector COVID-19 Vaccine for intranasal Spray (DelNS1-2019-nCoV-RBD-OPT1)

A Multicenter, Randomized, Double-Blind, Placebo-Controlled Phase 2 Clinical Trial of MW33 Injection to Evaluate the Efficacy and Safety in Patients With Mild or Moderate COVID-19

Safety and Immunogenicity of SARS-CoV-2 mRNA Vaccine (BNT162b2) in Chinese Healthy Population: A Phase II, Randomized, Placebo-controlled, Observer-blinded Study

Safety and Immunogenicity of a Recombinant COVID-19 Vaccine (CHO Cell) in Healthy Population Aged 18 Years and Older: A Phase I Study

A Single-center, Randomized, Double-Blinded, Placebo-Controlled, Phase II Clinical Trial of Recombinant COVID-19 Vaccine (Sf9 Cells), in the Subjects From Healthy Aged 18 Years and Above

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COVID-19 DNA vaccine phase II clinical trial

A randomized, double-blind, placebo-controlled phase III clinical trial of the effectiveness and safety of inoculation of recombinant new coronavirus vaccine (CHO cells) in the prevention of COVID-19 in people 18 years and older

Efficacy and Safety of High-dose Vitamin C Combined With Traditional Chinese Medicine in the Treatment of Moderate and Severe Coronavirus Pneumonia (COVID-19)

A clinical study on the treatment of severe COVID-19 patients with a new aerosol inhalation agent

A Phase 1 Randomized, Single-blind, Placebo-controlled, Single Ascending Dose Escalation Study of the Safety, Tolerability, and Pharmacokinetics of Human Monoclonal Antibodies, Brii-196 and BRII-198, Administered Intravenously to Healthy Adult Volunteers

A randomized, controlled, multicenter study on the safety and immunogenicity of SARS-CoV-2 inactivated vaccine

A randomized, controlled, multicenter study on the safety and immunogenicity of SARS-CoV-2 inactivated vaccine

A randomized, controlled, multicenter study on the safety and immunogenicity of SARS-CoV-2 inactivated vaccine

A Phase II clinical trial to evaluate the immunogenicity and safety of different doses of a novel coronavirus pneumonia (COVID-19) mRNA vaccine in population aged 18-59 years

A randomized, open-label, blank-controlled, multi-center clinical study for Lian-Hua Qing-Ke Tablets in the treatment of severe novel coronavirus pneumonia

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A randomized, open-label, blank-controlled, multi-center clinical study for Lian-Hua Qing-Ke Tablets in the treatment of COVID-19 patients with mild and common-type

A randomized, open-label, blank-controlled, multi-center clinical study for Lian-Hua Qing-Wen Capsule/Granule in the treatment of asymptomatic patients with novel coronavirus pneumonia

Phase I clinical study to evaluate the safety, tolerability, pharmacokinetic profile and immunogenicity of JMB-2002 injection in Chinese healthy subjects after intravenous infusion of single dose

A Single-center, Randomized, Double-Blinded, Placebo-Controlled, Phase IIb Clinical Trial of Recombinant COVID-19 Vaccine (Sf9 Cells), in the Subjects From Healthy Aged 18 to 85 Years Old

Qing-Jin Yi-Qi granule in the intervention of patients in rehabilitation phase of COVID-19

Real-world study on the prevention effect of Lian-Hua Qing-Wen Capsules on close contacts of new coronavirus pneumonia

A randomized, double-blind, placebo-controlled trial of Shugan Jieyu Capsules for improving sleep and mood disorders during recovery from novel coronavirus pneumonia

A Phase 2 Study to Evaluate the Safety and Efficacy of Human Monoclonal Antibodies, BRII-196 and BRII-198, Administered by Intravenous Infusion for the Treatment of COVID-19 Patients

Comprehensive evaluations of dynamic changes in the immune system of healthy volunteers after vaccination with inactivated Novel Coronavirus Pneumonia (COVID-19) vaccine (Vero cells) (SARS-CoV-2 Vaccine (Vero Cell), Inactivated)

A Study to Evaluate the Safety and Immunogenicity of Concomitant Administration of the SARS-CoV-2 Inactivated Vaccine (Vero Cell) With Quadrivalent Influenza Vaccine in Adults Aged From 18 to 59 Years

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A Single-center, Randomized, Double Blinded, Placebo Controlled, Phase 2 Clinical Trial of Recombinant COVID-19 Vaccine (CHO Cells), in the Subjects From Healthy Aged 18 Years and Above

Clinical study on the treatment of COVID-19 by Qingfei Paidu Granule

Safety and Immunogenicity of a Heterologous Immunization of Recombinant COVID-19 Vaccine (Ad5 Vector) and RBD-based Protein Subunit Vaccine Against COVID-19 in Chinese Healthy Population

A Randomized, Double-blind, Placebo-controlled Phase II Clinical Trial to Evaluate the Immunogenicity and Safety of Recombinant SARS-CoV-2 Fusion Protein Vaccine (V-01) in Healthy Subjects

A Randomized, Double-blind, Placebo-controlled Phase I Clinical Trial to Evaluate the Safety and Immunogenicity of Recombinant SARS-CoV-2 Fusion Protein Vaccine (V-01) in Healthy Subjects

The Impact of Timing of Vaccination on the Immune Reaction to Vaccine against SARS-CoV-2

A Randomized, Double-blind, Placebo-Controlled Phase I/II Clinical Trial to Evaluate the Safety and Immunogenicity of Ad5-nCoV for Inhalation in Adults 18 Years of Age and Older

Phase I/II Clinical Trial to Evaluate the Safety, Tolerability and Immunogenicity of Recombinant SARS-CoV-2 Vaccine (CHO Cell) in Healthy People Aged 3 Years and Older

A Immunobridging and Immunization Schedules Study of COVID-19 Vaccine (Vero Cell), Inactivated in Population Aged = 60 Years Old and 3-17 Years Old Compared With That in Population Aged 18-59 Years Old

Randomized, double-blind, placebo-controlled phase I clinical trial to evaluate the safety and immunogenicity of mRNACOVID-19 vaccine in healthy susceptible populations aged 18 years and older people

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A Randomized, Double-Blinded, Placebo-Controlled, Phase IIb Clinical Trial, to Evaluate the Safety of the Inactivated SARS-CoV-2 Vaccine (Vero Cell) in Healthy Population Aged From 3 to 17 Years.

A Randomized, Controlled, Multicenter Phase 4 Clinic Trial to Evaluate the Immunogenicity and Safety of Combined Immunization of Inactivated SARS-CoV-2 Vaccines (Vero Cell) (COVIV) and 23-valent Pneumococcal Polysaccharide Vaccine (PPV23) or Quadrivalent Inactivated Influenza Vaccine (IIV4)

A Randomized, Controlled, Multi-center Clinical Study To Evaluate The Immunogenicity And Safety Of Combined Immunization of Inactivated SARS-CoV-2 Vaccines (Vero Cell) (COVIV) and 23-valent Pneumococcal Polysaccharide Vaccine (PPV23) or Quadrivalent Inactivated Influenza Vaccine (IIV4)

A Randomized Controlled Trial for Qing Jin Yi Qi granule in the treatment of Convalescence Patients of Novel Coronavirus Pneumonia(COVID-19)

Efficiency and Safety of Nicotinamide-based Supportive Therapy in Lymphopenia for Patients With COVID-19: A Randomized Clinical Trial

To Evaluate the Immunogenicity Bridging Between Different Manufacture Scales and Between Different Lots of Novel Coronavirus Vaccine (Adenovirus Type 5 Vector) (Ad5-nCoV) in Population 6-59 Years of Age.

Safety and Immunogenicity of Sequential Immunization of Inactivated SARS-CoV-2 Vaccine and Recombinant SARS-CoV-2 Vaccine (Ad5 Vector) in Chinese Healthy Adults Aged 18-59 Years: a Randomized, Observer-blind, Parallel-controlled Clinical Trial

An extended Clinical Trial of Influenza virus Vector COVID-19 Vaccine for intranasal Spray (DelNS1-2019-nCoV-RBD-OPT1)

Comprehensive evaluations of dynamic changes in the immune system of healthy volunteers with sequential vaccination of inactivated Novel Coronavirus Pneumonia vaccine (Vero cells) followed by Novel Coronavirus Pneumonia mRNA vaccine (DF104B1)

A randomized, double-blind, placebo-controlled trial for Xinguan Fuzheng Zhuanyin Decoction to reduce the viral load of the delta variant of COVID-19

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A Multi-center, Double-blind, Randomized, Controlled, Phase 2 Study to Evaluate the Immunogenicity and Safety of CpG 1018/Alum-adjuvanted Recombinant SARS-CoV-2 Trimeric S-protein Subunit Vaccine (SCB-2019) in Individuals Aged 18 Years and Older in China

A Phase I, Single Ascending Dose Study to Evaluate the Safety, Tolerability, Pharmacokinetics, and Immunogenicity of LY-CovMab Injection in Chinese Healthy Subjects

Immunogenicity and Safety of an Inactivated COVID-19 Vaccine for Prevention of COVID-19 in Healthy Population Aged 18 Years and Older

A Randomized, Blinded, Placebo-controlled Phase I Clinical Trial of Recombinant New Coronavirus Vaccine (CHO Cells) in the Safety and Tolerability of Healthy People Aged 3 to 17 Years of Age

A Randomized, Double-blinded, Placebo-controlled Phase I Clinical Trial to Evaluate the Safety, Tolerability, and Preliminary Immunogenicity of the Recombinant SARS-CoV-2 Vaccine (CHO Cell) in Healthy Adults Aged 18 Years and Above

A Randomized, Double-blinded, Placebo-controlled Phase II Clinical Trial to Evaluate the Immunogenicity and Safety of the Recombinant SARS-CoV-2 Vaccine (CHO Cell) in Healthy Adults Aged 18 Years and Above

A Randomized and Controlled IV Clinical Trial to Evaluate the Immunogenicity and Safety of COVID-19 Vaccine (Vero Cell), Inactivated Co-administration With Quadrivalent Influenza Vaccine and 23-valent Pneumococcal Polysaccharide Vaccine

A Randomized, Double-blind, Placebo-controlled Phase II Clinical Study to Evaluate the Safety and Immunogenicity of Inactivated SARS-CoV-2 Vaccine (Vero Cells) in Healthy Population Aged 3 to 17 Years

A Randomized, Double-blind, Placebo-controlled Phase I Clinical Study to Evaluate the Safety and Immunogenicity of SARS-CoV-2 Vaccine (Vero Cells), Inactivated in Healthy Population Aged 3 to 17 Years

A Double-blind, Randomized Clinical Trial to Evaluate the Lot-to-lot Consistency, Immunogenicity and Safety of an Inactivated SARS-CoV-2 Vaccine (CoronaVac) in Health Adults Aged 26-45 Years

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Single-center, Randomized, Double-blind, Placebo-controlled Phase I/II Clinical Trial to Evaluate the Safety, Tolerability and Immunogenicity of Recombinant COVID-19 Vaccine (Sf9 Cells) in Healthy People Aged 6-17 Years

A randomized, double-blind, placebo-controlled phase I clinical trial to evaluate the safety and immunogenicity of the Novel Coronavirus inactivated vaccine (Vero cells) in healthy individuals 18 years of age and older

Study on Chinese Medicine Rehabilitation Promotion Program for COVID-19 Patients during Convalescence

Treatment Based on Syndrome Differentiation for 'Three formulations' in Delta novel coronavirus pneumonia (COVID-19): A Multicentre, Open-Label, Randomised Controlled Clinical Trial

Evaluation of clinical efficacy of Chinese medicine in promoting nucleic acid negative transformation of novel coronavirusDelta mutant strain

Study on Prevention and Treatment of Novel Coronavirus Pneumonia (COVID-19) With Traditional Chinese Medicine

A Phase II Clinical Trial to Evaluate the Immunogenicity and Safety of the Recombinant SARS-CoV-2 Vaccine (Chimpanzee Adenoviral Vector, ChAdTS-S) in Participants Aged 18 to 59 Years as well as 60 Years and Older

A Phase I/II Clinical Trial to Evaluate the Immunogenicity and Safety of the SARS-CoV-2 mRNA Vaccine in Healthy Population Aged 60 Years and Older

A Pragmatic, Randomized Clinical Trial of Coronavirus (COVID-19) Vaccinations in Uveitis

Phase I clinical trial to evaluate the safety, tolerability and preliminary immunogenicity of the new coronavirus mRNA vaccine (LVRNA009) in Chinese people aged 18 years and over

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A Global, Multi-center, Randomized, Double-Blind, Placebo-controlled, Phase III Clinical Study to Evaluate the Protective Efficacy, Safety and Immunogenicity of SARS-CoV-2 mRNA Vaccine in Population Aged 18 Years and Older

Rehabilitation for discharged COVID-19 patients: a randomized controlled clinical study

Randomized, Open, Multicenter Study on the Efficacy and Safety of Arbidol Hydrochloride Tablets in Treating Pneumonia in Patients Infected With Novel Coronavirus (2019-ncov).

An IIT, Randomized, Single-Center, Open-Lable, Standard Therapy Controlled Study to Evaluate the Efficacy and Safety of Colchicine Tablets in Patients With COVID-19

Medium-and long-term effects of Liu Zi Jue combined with Acupressure Therapy on pulmonary function and quality of life of discharged patients with severe COVID-19

Immunogenicity and Safety of the Heterologous Prime-boost Immunization With an Aerosolised Adenovirus Type-5 Vector-based COVID-19 Vaccine (Ad5-nCoV) After Two-dose Priming With an Inactivated SARS-CoV-2 Vaccine in Adults at 18 Years of Age or Above: a Randomised, Open-label, Parallel-controlled Clinical Trial

Rehabilitation assessment and treatment for Discharged COVID-19 patients

Safety and Immunogenicity of Sequential Immunization of Inactivated COVID-19 Vaccine and Recombinant COVID-19 Vaccine (Ad5 Vector) inChinese Healthy Adults Aged 60 and Above: a Randomized, Observer-blind,Parallel-controlled Clinical Trial

An Open-label,Phase IV Clinical Trial to Evaluate the Immunogenicity and Safety of the Inactivated SARS-CoV-2 Vaccine (Vero Cell) in Healthy Population Aged From 18 to 59 Years.

A Randomized, Controlled, Multicenter Phase 4 Clinic Trial to Evaluate the Immunogenicity and Safety of Combined Immunization of Inactivated SARS-CoV-2 Vaccines (Vero Cell) (COVAX) (Produced in Wuhan) and 23-valent Pneumococcal Polysaccharide Vaccine (PPV23) or Quadrivalent Inactivated Influenza Vaccine (IIV4)

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Efficacy and Safety of a Recombinant Neutralizing Human Anti-SARS-CoV-2 Monoclonal Antibody JS016 in Chinese Hospitalized Patients With SARS-CoV-2 Infection (COVID-19): a Multicenter, Randomized, Open-label, Controlled Trial

A Randomized, Double-Blinded Clinical Trial to Evaluate Additional Dose of Medium-dosage or High-dosage SARS-CoV-2 Inactivated Vaccine in Healthy Individuals Previously Vaccinated With CoronaVac

Randomized, Double-blind, Parallel-controlled Evaluation of the Immunogenicity Consistency and Safety of the Three Batches of Recombinant Novel Coronavirus Vaccine (CHO Cells) After Process Verification in the 18-59-year-old Population, and the Comparison With the Pilot-scale Batch Immunogenicity Non-inferiority Bridging and Safety Clinical

Follow-up investigation of COVID-19 lung function and rehabilitation training in discharged patients

An open-label, randomized, controlled clinical trial of Leflunomide in the treatment of COVID-19

Randomized, double-blind, placebo-controlled phase II clinical trial of immunogenicity, immunopersistence, and safety of graded Novel Coronavirus inactivated vaccine (Vero cells) in healthy persons aged 18 years and older

Open label study of the COVID-19 vaccine activating humoral immunity to promote nucleic acid negative conversion

A randomized, open-label, controlled clinical trial for azvudine in the treatment of new coronavirus delta

Clinical Study on the Combined Application of Hymecromone Tablets in the Treatment of SARS-CoV-2 Infection

Clinical Study on Immunogenicity and Safety of Recombinant Novel Coronavirus(COVID-19)Vaccine (CHO Cell) Combined With Tetravalent Influenza Virus Lysis Vaccine in People Aged 18 Years and Over

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A Phase IIIb Clinical Trial to Evaluate the Immunogenicity and Safety of One Heterologous Booster Dose of SARS-CoV-2 mRNA Vaccine in Population Aged 18-59 Years and 60 Years and Above Who Have Completed the 2-dose Series with Inactivated COVID-19 Vaccines

A Randomized and Controlled Phase IV Clinical Trial to Evaluate the Immunogenicity and Safety of COVID-19 Vaccine (Vero Cell), Inactivated Co-administrated With EV71 Vaccine (Vero Cell) in Children Aged 3-5 Years Old

A randomized, double-blind and placebo-controlled phase I/II clinical trial to explore the safety and immunogenicity of the COVID-19 DNA vaccine used as a boost dose to subjects previously immunized with COVID-19 inactivated vaccines

To evaluate the clinical mechanism of Lianhua Qingwen Capsule on the systemic immune regulation of patients with new coronavirus pneumonia

A Double-blind, Randomized Clinical Trial to Evaluate the Lot-to-lot Consistency, Immunogenicity and Safety of an Inactivated SARS-CoV-2 Vaccine (CoronaVac)Between Different Workshops for Prevention of COVID-19 in Healthy Children Aged 3-17 Years

Safety and Immunogenicity of Three Doses of an Inactivated SARS-CoV-2 Vaccine in Chinese Pulmonary Tuberculosis Patients Aged 18-75 Years: a Randomized, Double-blind, Parallel-controlled Clinical Trial

A Randomized, Open, Control Phase IV Clinical Trial, to Evaluate One Booster Immunization of Inactivated COVID-19 Vaccine (CoronaVac) or Original Vaccine in Adults Aged 18-45 Years Old Previously Vaccinated With Inactivated COVID-19 Vaccine From Difference Manufactures

A Randomized, Double-Blinded Clinical Trial to Evaluate the Immunogenicity Using Additional Dose of Medium-dosage or High-dosage COVID-19 Vaccine (Vero Cell), Inactivated in Populations Who Have Completed Primary Immunization 5-9 Months

A Safety, Tolerability, and Efficacy Study of IBI314 in Mild to Moderate Patients With COVID-19

A Phase I Clinical Trial to Evaluate the Safety, Tolerability and Preliminary Immunogenicity of the Candidate Recombinant SARS-CoV-2 Vaccine ChAdTS-S in Participants Aged 18 to 59 Years and Above

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A Randomized, Double-Blind, Placebo-Controlled Phase 1 Clinical Trial to Evaluate the Tolerability, Safety, Pharmacokinetic Profiles, and Immunogenicity of JS026 and JS026 in Together With JS016 Administered Intravenously to Healthy Chinese Subjects

Efficacy and Safety of Oral Lactobacillus Plantarum GUANKE (CGMCC NO.21720) in Enhancement of Antibody Level After SARS-CoV-2 Vaccination: A Prospective, Randomized, Double-blind, Placebo-Controlled Trial (Trial 1)

Efficacy and Safety of Oral Lactobacillus Plantarum GUANKE (CGMCC NO.21720) in Enhancement of Antibody Level After SARS-CoV-2 Vaccination: A Prospective, Randomized, Double-blind, Placebo-Controlled Trial (Trial 2)

A randomized, double-blind, placebo-controlled, multicenter clinical study of Jinshuibao in improving the symptoms related to pulmonary and cardiac function of COVID-19 during convalescence

Efficacy, safety and immunogenicity of a recombinant protein subunit vaccine (CHO cells) against COVID-19 in adults: an international multicenter, randomized, double-blind, placebo-controlled phase III study

A prospective, randomized, open, comparative, multi-center controlled trial to evaluate the safety, efficacy and immunogenicity of the third dose of vaccine reactivation in subjects that had received two doses of inactivated COVID-19 vaccine

SARS-CoV-2 mRNA Vaccine Heterologous Booster Immunization Strategy for Exploratory Clinical Trials (Yunnan)

Mindfulness Effects in COVID-19 Patients During the Coronavirus Disease 2019 (COVID-19) Pandemic: A Randomized Controlled Trial

A randomized, double-blind, placebo-controlled trial of a Phase II clinical study evaluating different doses of COVID-19 DNA vaccine in healthy adult and elderly subjects

Heterologous Prime-boost Immunization With an Aerosolised Adenovirus Type-5 Vector-based COVID-19 Vaccine (Ad5-nCoV) After Two-dose Priming With an Inactivated SARS-CoV-2 Vaccine in Adults Aged 18 Years and Above: a Multicenter, Open-label, Partially Parallel-controlled Clinical Trial

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A Randomized, Double-Blind, Placebo-Controlled, Bridging Study to Evaluate the Safety, Tolerability, and Pharmacokinetics of FB2001 in Healthy Subjects

Reactogenicity and Immunogenicity of Third Dose Vaccine Booster Following Two Doses of Inactivated Vaccines

A Randomized, Double-blind, Placebo-controlled Phase I/II Clinical Study to Evaluate the Safety, Tolerability and Immunogenicity of SCTV01C in Healthy Population Aged ≥18 Years Previously Unvaccinated Against COVID-19

A Phase II Double-blind, Placebo-controlled Study to Evaluate the Safety and Tolerability of AZD7442 in Chinese Adults

A study on the compliance and effectiveness of nursing exercise intervention on prone positioning in patients with mild and common COVID-19

The Respiratory Training for COVID-19 Patients: A Randomized Controlled Trial Study

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Primary Purpose	Suspension Flag
Treatment	0
Treatment	0
Treatment	0
Treatment	0
Treatment	0
Treatment	0
Treatment	0
Treatment	1
Treatment	0

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Treatment	1
Treatment	1
Treatment	0
Treatment	0
Treatment	0
Prevention	0
Treatment	0
Treatment	0
Prognosis/Quality of life	0
Prognosis/Quality of life	0

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Treatment	0
Prognosis/Quality of life	0
Prognosis/Quality of life	0
Treatment	0
Treatment	0
Treatment	0
Treatment	0
Treatment	0
Treatment	0
Treatment	0

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Treatment	0
Treatment	0
Treatment	0
Treatment	0
Treatment	0
Treatment	1
Treatment	0
Treatment	1
Treatment	1
Treatment	0

For peer review only

Treatment	0
Treatment	0
Treatment	0
Prevention	0
Treatment	0
Treatment	0
Treatment	0
Treatment	0
Treatment	0
Treatment	0

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Treatment	0
Treatment	0
Treatment	0
Treatment	0
Treatment	0
Treatment	0
Treatment	0
Treatment	0
Treatment	1
Treatment	1

For peer review only

Treatment	1
Treatment	1
Treatment	0
Treatment	0
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Treatment	0

For peer review only

Treatment	0
Treatment	0
Treatment	0
Treatment	0
Prevention	1
Treatment	0
Treatment	0
Treatment	0
Treatment	1
Treatment	0

For peer review only

Treatment	0
Treatment	1
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Treatment	0

For peer review only

Treatment	0
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Treatment	0
Treatment	0
Treatment	0
Prevention	0
Treatment	0
Treatment	0

For peer review only

Treatment	0
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Treatment	0
Treatment	0
Treatment	0

For peer review only

Treatment	0
Treatment	0
Treatment	0
Treatment	0
Treatment	0
Treatment	1
Treatment	0
Prognosis/Quality of life	1
Treatment	0
Treatment	0

For peer review only

Treatment	1
Prevention	0
Treatment	0
Treatment	0
Treatment	1
Treatment	0
Treatment	0
Treatment	0
Treatment	1
Treatment	0

For peer review only

Treatment	0
Treatment	0
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Treatment	1
Treatment	1
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Treatment	0
Treatment	0

For peer review only

Treatment	0
Treatment	0
Treatment	0
Treatment	0
Treatment	0
Treatment	1
Prognosis/Quality of life	0
Treatment	0
Treatment	0
Treatment	1

For peer review only

Treatment	0	
Treatment	0	
Treatment	0	
Treatment	0	
Treatment	0	
Treatment	0	
Treatment	0	
Treatment	0	
Treatment		0
Prevention	0	

For peer review only

Treatment	1
Treatment	1
Treatment	0
Treatment	1
Treatment	0
Treatment	0
Treatment	1
Treatment	0
Treatment	0
Prevention	0

For peer review only

Treatment	1
Treatment	0
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Treatment	1

For peer review only

Treatment	0
Treatment	0
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Treatment	0
Treatment	0
Treatment	0
Prognosis/Quality of life	0
Treatment	0

For peer review only

Treatment	0	
Treatment		0
Prevention	0	
Treatment	0	
Treatment	0	
Treatment	0	
Treatment	0	
Treatment	0	
Treatment	0	
Treatment	0	

For peer review only

Prognosis/Quality of life	0
Prevention	0
Treatment	0
Treatment	0
Treatment	0
Treatment	1
Treatment	0
Treatment	0
Treatment	0
Treatment	0

For peer review only

Treatment	0
Treatment	0
Prognosis/Quality of life	0
Treatment	0
Treatment	0
Treatment	0
Treatment	0
Treatment	0
Treatment	0
Treatment	0

For peer review only

Treatment	0	
Treatment	0	
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Treatment		0
Treatment	0	

For peer review only

Treatment	0
Treatment	0
Treatment	0
Health Services Research	0
Treatment	0
Prevention	0
Treatment	0
Prevention	0
Prognosis/Quality of life	0
Treatment	0

For peer review only

Prevention	0
Prevention	0
Prevention	0
Treatment	0
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Treatment	0

For peer review only

Treatment	0
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Prevention	0
Prevention	0
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Treatment	0

For peer review only

Treatment	0
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Prevention	0
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Treatment	0
Treatment	0
Treatment	0
Prevention	0
Prognosis/Quality of life	0

For peer review only

Prognosis/Quality of life	0
Treatment	0
Treatment	0
Treatment	0
Treatment	0
Treatment	0
Prevention	0
Treatment	0
Prevention	0
Prevention	0

For peer review only

Prevention	0	
Treatment	0	
Treatment	0	
Treatment	0	
Treatment	0	
Treatment	0	
Prevention		0
Prevention		0
Treatment	0	
Prevention		0

For peer review only

Prognosis/Quality of life	0
Prevention	0
Prevention	0
Prevention	0
Treatment	0
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Prevention	0
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Treatment	0
Prevention	0

For peer review only

Treatment	0	
Prevention	0	
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For peer review only

Prevention	0	
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Prevention	0	
Treatment	0	

For peer review only

Treatment	0
Treatment	0
Treatment	0
Prevention	1
Treatment	0
Prevention	0
Treatment	0
Treatment	0
Prevention	0
Prevention	0

For peer review only

Prevention	0
Treatment	0
Prevention	0
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Prevention	0
Prevention	0
Prevention	0

For peer review only

Prevention	0	
Prevention		1
Prevention	0	
Treatment	0	
Treatment	0	
Prevention	0	
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Prevention	0	
Prevention	0	
Treatment	0	

For peer review only

Prevention	0
Treatment	0
Prevention	0
Prevention	0
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Prevention	0
Prevention	0
Prevention	0
Prevention	0

For peer review only

Prevention	0
Prevention	0
Prognosis/Quality of life	0
Treatment	0
Treatment	0
Prevention	0
Prevention	0
Prevention	0
Prevention	0
Prevention	0

For peer review only

Prevention	0
Prognosis/Quality of life	0
Treatment	0
Treatment	0
Prognosis/Quality of life	0
Prevention	0
Prognosis/Quality of life	0
Prevention	0
Prevention	0
Prevention	0

For peer review only

Treatment	0
Prevention	0
Prevention	0
Prognosis/Quality of life	0
Treatment	0
Prevention	0
Treatment	0
Treatment	0
Treatment	0
Prevention	0

For peer review only

Prevention	0
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Prevention	0

For peer review only

Treatment	0
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Treatment	0
Prevention	0
Prevention	0

For peer review only

Treatment	0
Prevention	0
Prevention	0
Treatment	0
Treatment	0
Prognosis/Quality of life	0

For peer review only

Reason for Suspension (If any)	Recruitment status at last submission
#NOTREQ	Recruiting
#NOTREQ	Completed
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
Follow new disciplines on disease control from government	Withdrawn
#NOTREQ	Completed

For peer review only

The epidemic of COVID-19 has been controlled well at present, no eligible patients can be recruitted.	Suspended
The epidemic of COVID-19 has been controlled well in China, no eligible patients can be enrolled at present.	Terminated
#NOTREQ	Recruiting
#NOTREQ	Completed
#NOTREQ	Completed
#NOTREQ	Not yet Recruiting
#NOTREQ	Not yet Recruiting
#NOTREQ	Not yet Recruiting
#NOTREQ	Not yet Recruiting
#NOTREQ	Not yet Recruiting

For peer review only

#NOTREQ	Not yet Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting

For peer review only

#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
Canceled by the investigator	Not yet recruiting
#NOTREQ	Recruiting
the study was started in the second half of the outbreak, and the number of qualifying COVID-19 patients decreased with the control of the epidemic so that we had to stop our trial before reaching the predefined sample size	Terminated
Canceled by the investigator	Not yet recruiting
#NOTREQ	Recruiting

For peer review only

#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Completed
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting

For peer review only

#NOTREQ	Suspending
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
Retracted due to lack of patient	Suspending
Cancelled due to lack of patient	Suspending

For peer review only

Cancelled due to lack of patient	Recruiting
Cancelled due to lack of patient	Suspending
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting

For peer review only

#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
Canceled by the investigators	Suspending
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Suspending
Cancelled by the investigator	Not yet recruiting
#NOTREQ	Not yet recruiting

For peer review only

#NOTREQ	Active, not recruiting
Cancelled by the investigator	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Completed
#NOTREQ	Not yet recruiting
#NOTREQ	Completed

For peer review only

#NOTREQ	Completed
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting

For peer review only

#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting

For peer review only

#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
There were no eligible patients in the paticipated centers.	Suspended
#NOTREQ	Recruiting
Cancelled due to lack of patient	Suspending
#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting

For peer review only

Cancelled by the investigator	Suspending
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
Without CDE Approval	Withdrawn
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
Cancelled by the investigator	Suspending
#NOTREQ	Recruiting

For peer review only

#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
Cancelled by the investigator	Not yet recruiting
Cancelled by the investigator	Not yet recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting

For peer review only

#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Completed
Patients were transferred to designated hospitals for treatment as needed, the clinical trials cannot be conducted.	Withdrawn
#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
Cancelled by the investigator	Not yet recruiting

For peer review only

#NOTREQ	Not yet recruiting
#NOTREQ	Completed
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting

For peer review only

Cancelled by the investigator	Not yet recruiting
Cancelled by the investigator	Not yet recruiting
#NOTREQ	Not yet recruiting
Cancelled by investigator	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
Cancelled by the investigator	Suspending
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting

For peer review only

Cancelled by the investigator	Not yet recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
Cancelled by the investigator	Not yet recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
Cancelled by the investigator	Not yet recruiting

For peer review only

#NOTREQ	Not yet recruiting
#NOTREQ	Enrolling by invitation
Cancelled by the investigator	Not yet recruiting
#NOTREQ	Recruiting
Canceled by the investigator	Completed
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting

For peer review only

#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Suspending
#NOTREQ	Recruiting

For peer review only

#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
There were no eligible patients with CoVID-19 in the paticipated centers.	Suspended
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting

For peer review only

#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting

For peer review only

#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Active, not recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting

For peer review only

#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Active, not recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting

For peer review only

#NOTREQ	Active, not recruiting
#NOTREQ	Recruiting
#NOTREQ	Completed
#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Completed
#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting

For peer review only

#NOTREQ	Enrolling by invitation
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting

For peer review only

#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Active, not recruiting
#NOTREQ	Completed
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Completed
#NOTREQ	Recruiting
#NOTREQ	Completed

For peer review only

#NOTREQ	Not yet recruiting
#NOTREQ	Completed
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Active, not recruiting
#NOTREQ	No longer recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Active, not recruiting

For peer review only

#NOTREQ	Enrolling by invitation
#NOTREQ	Completed
#NOTREQ	Completed
#NOTREQ	Completed
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Active, not recruiting
#NOTREQ	Completed
#NOTREQ	Completed
#NOTREQ	Active, not recruiting

For peer review only

#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Completed
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting

For peer review only

#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Completed
#NOTREQ	Recruiting
#NOTREQ	Completed
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Active, not recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting

For peer review only

#NOTREQ	Completed
#NOTREQ	Not yet recruiting
#NOTREQ	Active, not recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Completed
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting

For peer review only

#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
Sponsor decided to cancel this trial.	Withdrawn
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Completed
#NOTREQ	Recruiting
#NOTREQ	Completed

For peer review only

#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Active, not recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting

For peer review only

#NOTREQ	Active, not recruiting
Canceled by the investigator	Suspending
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Completed
#NOTREQ	Recruiting
#NOTREQ	Active, not recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting

For peer review only

#NOTREQ	Not yet recruiting
#NOTREQ	Active, not recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Active, not recruiting

For peer review only

#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting

For peer review only

#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Completed
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Active, not recruiting

For peer review only

#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting

For peer review only

#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Active, not recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Active, not recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting

For peer review only

#NOTREQ	Active, not recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Completed
#NOTREQ	Recruiting
#NOTREQ	Completed
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting

For peer review only

#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Completed

For peer review only

Study Start Date (Date of First Enrollment)	Estimated Total Enrollment	Number of Arms
2020/1/10	160	2
2020/1/26	80	2
2020/1/22	150	2
2020/1/29	48	2
2020/1/25	108	3
2020/1/30	30	2
2020/2/3	42	2
2020/2/5	40	2
2020/1/28	125	3

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2020/2/12	308	2
2020/2/6	452	2
2020/1/29	45	3
2020/2/1	240	2
2020/2/1	284	2
2020/2/1	160	8
2020/2/4	100	2
2020/2/2	120	2
2020/2/3	100	2
2020/2/2	100	2

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2020/2/3	100	2
2020/2/3	100	2
2020/2/3	90	3
2020/2/1	100	2
2020/2/1	400	4
2020/2/1	100	2
2020/1/29	90	3
2020/2/4	100	2
2020/2/4	140	4
2020/2/4	328	2

For peer review only

2020/2/10	100	3
2020/2/4	30	3
2020/2/4	30	3
2020/2/3	400	2
2020/1/31	300	3
2020/2/5	30	2
2020/2/1	60	3
2020/2/14	140	2
2020/2/5	30	2
2020/2/6	480	4

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2020/1/31	70	2
2020/2/6	120	2
2020/2/1	400	2
2020/2/1	600	2
2020/2/6	160	2
2020/2/6	400	4
2020/2/6	30	2
2020/1/25	63	5
2020/2/10	100	2
2020/2/14	100	2

For peer review only

2020/2/10	60	2
2020/2/11	78	2
2020/2/12	112	2
2020/2/10	90	5
2020/2/11	200	2
2020/2/15	120	2
2020/2/15	238	2
2020/2/14	200	2
2020/2/15	60	3
2020/2/12	240	2

For peer review only

2020/2/13	240	4
2020/2/12	60	2
2020/2/13	408	2
2020/2/10	188	2
2020/2/12	60	2
2020/2/15	40	2
2020/2/11	40	2
2020/2/5	160	2
2020/2/14	160	2
2020/2/14	160	2

For peer review only

2020/3/1	60	2
2020/2/15	100	2
2020/2/17	120	2
2020/2/15	30	2
2020/2/20	320	4
2020/2/12	120	3
2020/2/14	72	2
2020/2/7	110	2
2020/2/17	45	2
2020/4/20	48	2

For peer review only

2020/2/16	60	2
2020/2/17	120	2
2020/2/1	60	2
2020/2/19	68	2
2020/2/16	20	2
2020/2/15	180	3
2020/2/15	520	2
2020/2/11	360	2
2020/2/10	300	2
2020/2/14	400	2

For peer review only

2020/4/23	142	2
2020/2/17	100	2
2020/2/17	100	2
2020/2/10	100	2
2020/3/1	200	2
2020/3/1	200	2
2020/4/30	300	3
2020/2/18	120	2
2020/2/17	40	2
2020/2/9	300	2

For peer review only

2020/2/21	186	2
2020/2/13	80	2
2020/2/20	300	2
2020/1/30	120	2
2020/2/18	72	3
2020/2/17	100	3
2020/2/20	40	2
2020/2/19	200	2
2020/2/15	240	2
2020/2/15	60	2

For peer review only

2020/2/19	480	3
2020/2/19	60	2
2020/2/3	200	2
2020/2/19	100	2
2020/2/4	60	2
2021/10/1	300	3
2020/2/20	60	3
2020/2/19	300	2
2020/2/22	100	2
2020/2/24	40	2

For peer review only

2020/2/20	120	2
2020/2/24	828	3
2020/2/7	132	2
2020/2/21	300	2
2020/2/1	24	2
2020/2/22	100	3
2020/2/10	460	2
2020/3/10	200	2
2020/2/23	40	2
2020/2/24	180	2

For peer review only

2020/3/1	40	2
2020/2/28	60	2
2020/2/24	180	6
2020/2/1	16	2
2020/2/15	348	2
2020/2/24	120	2
2020/2/25	39	2
2020/2/24	60	2
2020/2/25	20	2
2020/3/2	80	2

For peer review only

2020/2/17	60	2
2020/2/24	100	2
2020/2/25	60	2
2020/2/15	120	2
2020/3/5	45	2
2020/2/24	48	2
2020/2/24	60	2
2020/2/26	120	6
2020/1/22	80	3
2020/2/26	32	4

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2020/2/22	186	2	
2020/2/20	240	2	
2020/3/1	300	2	
2020/2/22	60	2	
2020/2/27	20	2	
2020/3/1	26	2	
2020/2/24	30	3	
2020/2/27	204	2	
2020/3/4	292		2
2020/3/1	300	2	

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2020/3/2	60	2
2020/3/5	90	3
2020/7/1	280	6
2020/2/29	40	2
2020/2/18	60	2
2020/2/27	340	2
2020/3/1	30	2
2020/3/1	80	2
2020/3/1	60	2
2020/3/2	80	2

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2020/3/2	70	2
2020/3/1	60	2
2020/2/27	96	2
2020/3/2	394	2
2020/3/3	200	2
2020/2/26	120	2
2020/2/26	100	2
2020/3/3	328	2
2020/3/1	200	3
2020/3/1	16	2

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2020/2/2	90	3
2020/3/5	60	2
2020/3/13	40	2
2020/2/29	60	2
2020/3/9	100	2
2020/2/20	100	2
2020/3/8	152	2
2020/2/4	300	3
2020/3/7	40	2
2020/2/1	60	2

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2020/2/1	30	2	
2020/3/8		150	3
2020/3/16	240	4	
2020/3/17	140	2	
2020/3/9	60	2	
2020/3/10	60	2	
2020/2/15	50	2	
2020/3/10	40	2	
2020/2/10	50	2	
2020/2/12	80	2	

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2020/3/1	120	2
2020/3/1	120	4
2020/2/15	70	2
2020/3/16	100	2
2020/2/1	128	2
2021/11/1	116	2
2020/3/6	40	2
2020/2/10	20	2
2020/2/20	100	4
2020/2/26	30	2

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2020/2/15	400	4
2020/3/17	60	2
2020/3/31	108	3
2020/3/19	144	2
2020/9/1	20	2
2020/3/5	150	3
2020/3/23	204	2
2020/3/18	60	2
2020/3/23	60	2
2020/2/26	40	2

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2020/3/23	1000	2	
2020/4/1	20	2	
2020/1/30	300	2	
2020/2/15	100		2
2020/2/21	90	5	
2020/3/1	#NaN	2	
2020/4/1	80	2	
2020/3/13	60	2	
2020/5/6		20	2
2020/4/1	80	2	

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2020/1/25	56	2	
2020/4/1	210	2	
2020/4/1	750	2	
2020/4/1	100	2	
2020/4/1	150	2	
2020/4/12		500	3
2020/3/1	20	2	
2020/4/11	1456	44	
2020/4/20	120	2	
2020/2/11	300	2	

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2020/4/16	744	12
2020/4/16	2000	2
2020/4/1	3808	4
2020/4/17	45	3
2020/4/22	50	4
2020/5/10	60	2
2020/4/25	160	2
2020/4/15	200	2
2020/5/1	200	2
2020/4/24	350	2

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2020/2/10	128	2
2020/5/1	60	2
2020/3/1	242	2
2020/2/19	186	2
2020/4/20	200	2
2020/5/1	60	2
2020/4/29	2128	68
2020/3/26	2000	2
2020/5/2	120	2
2020/5/6	60	2

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2020/5/8	60	2
2020/5/12	342	2
2020/5/22	422	4
2020/2/8	#NaN	2
2020/5/7	240	2
2020/5/8	200	2
2020/2/25	160	2
2020/2/12	60	2
2020/5/15	942	8
2020/2/15	120	2

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2020/6/1	150	2
2020/2/15	60	2
2020/2/18	104	2
2020/1/21	300	2
2020/6/5	40	2
2020/3/2	30	2
2020/6/22	50	3
2020/2/10	93	2
2020/6/25	168	3
2020/7/12	900	6

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2020/7/10	471	4	
2020/7/12	12	3	
2020/7/13	12	3	
2020/7/24	33	2	
2020/5/1	60	2	
2020/11/13	32	4	
2020/8/19	50	3	
2020/7/28	144	6	
2020/8/7	32	2	
2020/8/28	168	12	

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2020/8/29	1600	2
2020/9/1	60	2
2020/9/14	45	2
2020/10/31	552	3
2020/10/1	120	2
2020/10/1	120	2
2020/9/24	481	9
2020/10/30	90	2
2020/8/1	200	2
2020/10/7	180	9

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2020/10/31	198	2	
2020/10/28	120	5	
2020/2/28	111	2	
2020/10/27	1000		12
2020/10/31	1040	3	
2020/11/17	720	4	
2020/12/29	150	3	
2020/12/4	960		2
2020/10/30	216	12	
2020/11/17	960		16

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2020/12/2	640	4	
2020/11/30	29000	2	
2020/2/6	#NaN		6
2021/1/1	20	2	
2021/1/5	24	2	
2020/9/29	360	3	
2021/1/2	360	3	
2021/1/5	360	3	
2021/1/11	420	4	
2021/1/1	20	2	

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2021/1/1	120	2
2021/1/1	120	2
2021/1/14	40	4
2021/2/1	0	4
2021/1/29	200	2
2021/2/4	2200	2
2020/7/1	200	2
2021/6/20	48	3
2020/12/28	16	2
2021/3/23	480	2

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2021/3/23	480	8
2021/4/1	80	2
2021/4/7	120	4
2021/3/28	880	4
2021/2/22	180	6
2021/4/9	472	2
2021/4/20	840	22
2021/4/25	3580	15
2021/4/29	4400	18
2021/3/26	240	8

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2021/5/3	500	2	
2021/5/12	1152	3	
2021/5/14	1404	3	
2021/1/29	300	2	
3/1/2020	24	2	
6/3/2021	2016	9	
5/25/2021	300	4	
7/3/2021	400	2	
6/21/2021	20	2	
6/18/2021	100	2	

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8/1/2021	800	2
11/10/2020	42	6
7/9/2021	270	2
7/3/2021	75	6
7/12/2021	144	8
7/31/2021	1056	8
9/1/2021	1320	3
9/1/2021	480	2
8/1/2021	84	2
5/11/2021	1080	3

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8/15/2021	600	8
8/16/2021	150	6
8/1/2021	180	3
8/7/2021	600	2
8/7/2021	100	2
8/5/2021	2560	2
8/4/2021	360	2
7/28/2021	320	3
8/2/2021	1314	2
7/28/2021	144	6

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	7/22/2021	2000	2
	7/1/2021	102	3
2/8/2020		236	2
9/6/2021		60	2
2021/4/1		200	2
9/13/2021		420	3
2021/9/30		120	3
8/26/2021		300	4
6/19/2021		1400	3
5/6/2021		1404	3

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1/20/2021	200	2
11/1/2021	1200	2
9/9/2021	1680	4
2021/10/15	40	2
2021/10/1	60	2
2021/10/8	600	6
2021/11/3	60	2
2021/11/1	60	2
2020/7/25	300	2
9/3/2021	300	2

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2021/11/25	2000	4
11/5/2021	520	2
2021/12/25	480	6
2021/4/7	20	2
7/27/2021	2520	7
12/22/2021	240	3
12/20/2021	480	5
10/20/2021	340	2
12/31/2021	222	2
2021/5/24	60	2

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12/17/2021	48	7
3/1/2022	30	2
3/1/2022	40	4
2020/4/27	200	2
2021/9/2	14600	3
2021/8/2	200	3
2021/11/24	300	2
2021/10/16	66	2
2021/12/15	200	4
1/22/2022	10420	3

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12/29/2021	32	4
8/1/2021	400	4
12/1/2021	752	12
12/3/2021	272	2
2022/1/12	46	2
2020/1/1	89	2

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Arms

intervention group:Lopinavir-ritonavir tablets (each containing 200 mg of lopinavir and 50 mg of ritonavir), twice a day, 2 tablets at a time;Control group:Conventional standardized treatment;

Placebo Comparator: standard care
standard care
Intervention: Other: Standard care
Experimental: standard care + methylprednisolone therapy
Methylprednisolone 40 mg q12h for 5 days
Experimental: Conventional medicines and TCMs granules
Conventional medicines: oxygen therapy, antiviral therapy (alfa interferon via aerosol inhalation, and lopinavir/ritonavir, 400mg/100mg, p.o, bid) for 14 days.

Traditional Chinese Medicines (TCMs) granules: one bag, p.o, bid, for 14 days

Experimental group:Methylprednisolone, intravenous injection, 1-2mg/kgÂ·d for 3 days.;Control group:Without any glucocorticoid therapy;

arm A:Ribavirin + Interferon alpha-1b;arm B:lopinavir / ritonavir + interferon alpha-1b;arm C:Ribavirin + LPV/r+Interferon alpha-1b;

Experimental: Darunavir, Cobicistat and conventional treatments
After randomization, subjects take darunavir and cobicistat one tablet per day for 5 days, also take conventional treatments.
Intervention: Drug: Darunavir and Cobicistat
No Intervention: Conventional treatments

Experimental Group:Combined Treatment of Chinese medicine and western medicine;Control Group:Western medicine treatment;

Experimental: Observational group
5u washed microbiota suspension administered via nasogastric tube, nasojejunal tube or oral, combining with standard therapy.
Intervention: Other: washed microbiota transplantation
Placebo Comparator: Control group
Experimental: Group A-Standard treatment+lopinavir/ritonavir
In group A, 50 cases are given ordinary treatment plus a regimen of lopinavir (200mg) and ritonavir (50mg) (oral, q12h, every time 2 tablets of each, taking for 7-14 days).
Intervention: Drug: Lopinavir and Ritonavir Tablets
Active Comparator: Group B-Standard treatment+arbidol

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Experimental: Remdesivir group
active remdesivir
Intervention: Drug: Remdesivir
Placebo Comparator: Control group
Placebos matched remdesivir
Experimental: Remdesivir group
active remdesivir
Intervention: Drug: Remdesivir
Placebo Comparator: Control group
Placebos matched remdesivir

A:Critical Treatment in Critical Period + Ankylosaurus;B:Critical Treatment in Critical Period + Ankylosaurus+M1 suppression therapy;C:Critical Treatment in Critical Period;

Routine treatment + Lianhua Qingwen Capsules/granules:4 capsules / time or 1 bag / time, 3 times a day;Conventional treatment:Treat as prescribed in the fourth edition of the diagnosis and treatment plan;

Routine treatment + Lianhua Qingwen Capsules/granules:4 capsules / time, 3 times a day;Conventional treatment:Treat as prescribed in the fourth edition of the diagnosis and treatment plan, without using traditional Chinese medicine;

Community exposure TCM intervention group:TCM intervention;Community exposure control group:Placebo;Hospital exposure TCM intervention group:TCM intervention;Hospital exposure control group:Placebo;Community suspected TCM intervention group:TCM intervention;Community suspected control group:Placebo;Hospital suspected TCM intervention group:TCM intervention;Hospital suspected control

control group:western medical therapies;Experimental group:Conventional medicine + TCM;

control group:basic western medical therapies;experimental group:TCM standard decoctions+basic western medical therapies;

experimental group:Pulmonary rehabilitation+Conventional treatment;control group:Conventional treatment;

experimental group:shadowboxing +conventional treatment;Control group:conventional treatments;

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experimental group:TCM decoctions+basic conventional therapy;Control group:basic conventional therapies;

experimental group :TCM decoctions+basic western medical therapies ;control group :basic western medical therapies ;

experimental group:Traditional Chinese Medicine+psychological intervention; Control group:Traditional Chinese Medicine;Control group:psychological intervention;

Experimental: Abidol hydrochloride
Standard symptomatic support therapy (SMT) plus abidol hydrochloride(0.2g, 3 times a day).
Intervention: Drug: Abidol hydrochloride
Experimental: Abidol Hydrochloride combined with Interferon atomization
No intervention: Symptomatic supportive treatment
Symptomatic supportive treatment
Experimental: Abidol hydrochloride was added on the basis of group I.
Abidol hydrochloride 0.2g once,3 times a day,2 weeks
Intervention: Drug: Abidol hydrochloride
Experimental: Methylprednisolone(<40mg/d)
Intervention: Drug: Methylprednisolone
Experimental: Methylprednisolone(40~80mg/d)
Intervention: Drug: Methylprednisolone
Intervention A:On the basis of routine medical treatment, the patients were given Novaferon Atomization inhalation, 20ug each time, twice a day, 7-14 days.;intervention B:On the basis of routine medical treatment, the patients were given lopinavir / ritonavir tablets (Kaletra) twice a day, twice each time, 7-14 days.;intervention C:On the basis of routine medical treatment, combined with Novafron and Kaletra, the usage and course of

treament group:Chinese medicine decoction;placebo group:placebo;

(ordinary) Western medicine group:Use the (ordinary) western medicine treatment plan in "the diagnosis and treatment plan of new coronavirus infection pneumonia" issued by the National Health Commission of the fourth edition.;(ordinary) Chinese and Western Medicine Group:Use the (ordinary) TCM clinical prescription combined with the western medicine treatment plan in "the diagnosis and treatment plan of new coronavirus infection

experimental group:conventional standardized treatment and Lopinavir-Ritonavir;control group:Conventional standardized treatment;

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DRV/c group:DRV/c (800mg/150mg QD) + Conventional treatment containing thymosin (1.6 mg SC QOD) ;LPV/r group:LPV/r (400mg/100mg bid) + Conventional treatment containing thymosin (1.6 mg SC QOD);other group:Conventional treatment containing thymosin (1.6 mg SC QOD) ;

Experimental group 1:current antiviral treatment+Baloxavir Marboxil tablets;Experimental group 2:current antiviral treatment+fabiravir tablets;Control group:current antiviral treatment;

A:BaloxavirMarboxil:80mg on day1,80mg on day4;and 80mg on day7 as neccessary. No more than 3 times administration in total.BaloxavirMarboxil:80mg on day1,80mg on day4;and 80mg on day7 as neccessary. No more than 3 times administration in total.;B:Favipiravir: 600 mg tid with 1600mg first loading dosage for no more than 14 days ;C:Lopinavir-Ritonavir: 2# (200mg/50 mg), twice daily, for 14days ;

Experimental group:Western medicine routine treatment combined with traditional Chinese medicine treatment;Control group:Western medicine routine treatment;

experimental group 1:Hydroxychloroquine 0.1 oral 2/ day;experimental group 2:Hydroxychloroquine 0.2 oral 2/ day;Placebo control group:Starch pill oral 2/ day;

control group :Conventional treatment ;Experimental group :conventional treatment combined with umbilical cord mesenchymal stem cell conditioned medium group;

Experimental: ASC09F+Oseltamivir
Intervention: Drug: ASC09F+Oseltamivir
Experimental: Ritonavir+Oseltamivir
Intervention: Drug: Ritonavir+Oseltamivir
Experimental: Oseltamivir
Experimental: VC
12g Vitamin C+sterile water for injection; total volume: 50ml. 12ml/h; infusion pump ; q12h.
Intervention: Drug: VC
Placebo Comparator: Sterile water for injection
50ml water for injection, 12ml/h; infusion pump; q12h

control group:Conventional treatment;experimental group:conventional treatment combined with umbilical cord blood mononuclear cells group;

Mild Type Group A:antiviral therapy and Chinese medicine treatment;Mild Type Group B:antiviral therapy, Chinese medicine treatment, and Novaferon atomization;Common and Heavy Type Group C:antiviral therapy and Chinese medicine treatment;Common and Heavy Type Group C:antiviral therapy, Chinese medicine treatment, and Novaferon atomization;

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Experimental group:Ruxolitinib combined with mesenchymal stem cell;control group: Routine treatment;

control group:basic western medical therapies;experimental group:Reduning injection combined with basic western medical therapies;

Control group:Health education+Basic treatment of western medicine;Experimental group:Health education+Basic treatment of Western medicine+Dialectical?treatment of traditional Chinese medicine;

Experimental group:health education, follow-up condition management by team of family doctors, Chinese medicine treatment;Control group:health education, follow-up condition management by team of family doctors;

Experimental group:Conventional standardized treatment and ASC09/Ritonavir;Control group:Conventional standardized treatment and Lopinavir/Ritonavir;

Low dose group:Shuanghuanglian 2 bottles/time, 3 times a day; Routine treatment.;Medium dose of group:Shuanghuanglian 4 bottles/time, 3 times a day; routine treatment.;High dose of group:Shuanghuanglian 6 bottles/time, 3 times a day; routine treatment.;Control group:Routine treatment;

Experimental: Hydroxychloroquine and conventional treatments
After randomization, subjects take hydroxychloroquine 400mg per day for 5 days, also take conventional treatments.
Intervention: Drug: Hydroxychloroquine
No Intervention: Conventional treatments

experimental group A:Conventional treatment followed by Intravenous infusion of Human Menstrual Blood-derived Stem Cells preparations;control gorup A:Conventional treatment;Experimental Group B1:Artificial liver therapy+conventional treatment;Experimental Group B2:Artificial liver therapy followed by Intravenous infusion of Human Menstrual Blood-derived Stem Cells preparations+conventional treatment;Control

Treatment group:baseline antiviral treatment (lopinavir / ritonavir or abidol) and inhaled with rSIFN-co;Control group:baseline antiviral treatment (lopinavir / ritonavir or abidol) and inhaled with interferon-a;

control group:Standard treatment;experimental group:Standard treatment and methylprednisolone for injection;

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experimental group:high-fow therapy by nasal cannulae (HFNC);control group:bag- valve mask oxygenation (SMO);

hydroxychloroquine group:oral intake hydroxycholoroquine 0.2 twice a day;control group:conventional therapy;

experimental group:Chloroquine Phosphate;control group:Lopinavir / Ritonavir;

General patients treated with normal treatment:Normal Treatment;General patients experimental group:Normal Treatment plus Sodium Aescinate for Injection;Severe patients control group 1:Normal treatment plus hormonotherapy;Severe patients control group 2:Normal Treatment;Severe patients experimental group:Normal Treatment plus Sodium Aescinate for Injection;

Experimental group:Traditional Chinese Medicine;control group:Basic treatment of modern medicine;

experimental group:routine treatment, Jinyebaidu granule, po, 1-2 bags once, 3 times a day;;control group:routinel treatment, treatment according to the fifth version of the diagnosis and treatment protocol, no use of traditional Chinese medicine.;

experimental group:Xiyanping injection;control group:alpha-interferon;

Experimental group:Conventional treatment and convalescent plasma therapy;Control group:Conventional treatment;

control group 1:Lopinavir / Ritonavir (Kaletra) and IFN aerosol inhalation;control group 2:Abidol and IFN aerosol inhalation.;Experimental group:ASC09/ Ritonavir (ASC09F) and IFN aerosol inhalation;

Experimental group:Hydroxychloroquin;Control group:Lopinavir /Ritonavir;

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Low-dose group:Low-dose hydroxychloroquine and conventional therapy;Medium-dose group:Medium-dose hydroxychloroquine and conventional therapy;High-dose group:High-dose hydroxychloroquine and conventional therapy;control group:conventional therapy;

Experimental group:Conventional treatent and hydroxychloroquine;Control group:Conventional treatment;

experimental group:TCM and general treatment;control group:general treatment;

control group:conventional therapy;Experience group:conventional therapy+tocilizumab;

experimental group:Diammonium Glycyrrhizinate Enteric-coated Capsules (oral, 150mg, Tid), Vitamin C tablets (oral, 0.5g, QD) and clinical standard antiviral treatment;control group:clinical standard antiviral treatment;

Experimental group:Routine treatment + Babaodan 6 capsules, bid orally;Control group:Conventional treatment;

experimental group:Polyinosinic-Polycytidylic Acid Injection and conventional therapy;control group:conventional therapy;

experimental group:Truncation and Torsion Formula and Routine treatment of Western Medicine;control group:Routine treatment of Western Medicine;

experimental group:Routine treatment and Shenqi Fuzheng Injection;control group:Routine treatment ;

experimental group:routine treatment and Kangbingdu granules;control group:routine treatment ;

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Control group :Western medicine routine treatment plan;Intervention group :Western medicine routine treatment plan plus TCM syndrome differentiation treatment;

Experimental group:TCM based on symptomatic treatment;Control group:Placebo TCM based on symptomatic treatment;

Control group:Western medicine basic treatment;Experimental group:Western medicine basic treatment combined with traditional Chinese medicine;

Experimental: NK Cells Treatment Group
Conventional treatment plus NK cells. Participants will receive conventional treatment plus twice a week of NK cells (0.1-2*10E7 NK cells/kg body weight).
Intervention: Biological: NK Cells
No Intervention: Conventional Control Group

A1:Hydroxychloroquine, small dose;A2:Hydroxychloroquine, high dose;B1:Abidol hydrochloride, small dose;B2:Abidol hydrochloride, high dose;

Thymosin treatment group:Thymosin for injection 1.6 mg sc qd for 5 days;PD-1 treatment group:Camrelizumab 200 mg single dose, diluted to 100 ml intravenous infusion;Conventional treatment group:Conventional treatment;

Experimental group:Conventional Treatment & Tanreqing Capsules (oral, 3 capsules at a time, 3 times a day);Control Group:Conventional Treatment;

Experimental group:honeysuckle decoction;Control group:placebo;

Phosphoric chloroquine:2 tablets phosphoric chloroquine BID, complicated with therapy recommended by "New Coronavirus Infected Pneumonia Diagnosis and Treatment Scheme (revision of Trial Version 5)";Placebo:2 tablets placebo BID;

Experimental: UC-MSCs Treatment Group
Conventional treatment plus UC-MSCs:

Participants will receive conventional treatment plus 4 times of UC-MSCs(0.5*10E6 UC-MSCs/kg body weight intravenously at Day 1 , Day 3 , Day 5 , Day7)

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Experimental: Group A
Treatment group
Interventions:
Drug: Bromhexine Hydrochloride Tablets
Drug: Arbidol Hydrochloride Granules

Phosphoric chloroquine:2 tablets phosphoric chloroquine BID, complicated with therapy recommended by New Coronavirus Infected Pneumonia Diagnosis and Treatment Scheme (revision of Trial Version 5);Placebo:2 tablets placebo BID, complicated with therapy recommended by New Coronavirus Infected Pneumonia Diagnosis and Treatment Scheme (revision of Trial Version 5);

Experimental group:Regulating intestinal flora + conventional treatment;Control group:conventional treatment;

control group:conventional therapy and placebo;Experience group:conventional therapy and ALA;

Experimental group:Oral administration of 5 tablets of 1mg Azvudine tablets daily;Control group:According to the "Pneumonia Diagnosis and Treatment Plan for New Coronavirus Infection (Trial Version 5)" issued by National Health Commission of People's Republic of China, the subjects were given corresponding treatment.;

Chinese traditional medicine group 1:Traditional Chinese medicine qingfei prescription treatment;Chinese traditional medicine group 2:Traditional Chinese medicine clear lung prescription and compound houttuynia mixture treatment;Control group:Conventional treatment;

Experimental group:Carrimycin;Control group:Lopinavir and Ritonavir Tablets;

Experimental group:oral hydroxychloroquine sulfate tablets;Control group:Conventional treatment meet the Guideline;

experimental group:Truncated Torsion' Formula and Routine treatment of Western Medicine;control group:Routine treatment of Western Medicine;

Experimental: MP group
Intervention: Drug: Methylprednisolone
No Intervention: Con group

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Experimental: FZHY Group
usual treatment (respiratory function rehabilitation training + Vitamin C tablets)

Fuzheng Huayu tablets
Interventions:

hydroxychloroquine sulfate:Day1: first dose: 6 tablets(0.1g/table), second dose: 6 tablets (0.1g/table)after 6h ;Day2~5: 2 tablets (0.1g/table), BID;phosphate chloroquine:Day 1-3:500mg BID; Day 4-5:250mg BID;

hydroxychloroquine sulfate:Day1: first dose: 6 tablets(0.1g/table), second dose: 6 tablets (0.1g/table)after 6h ;Day2~5: 2 tablets (0.1g/table), BID;phosphate chloroquine:Day1-3:500mg,BID Day4-5:250mg,BID;

Conventional treatment group:The conventional treatment group will be treated according t o the guidance of the â€œDiagnosis and Treatment Scheme of COVID-19â€ published by the National Health Commission.;Experimental group:conventional treatment combined wit h Chloroquine Phosphate.;

experimental group :Traditional Chinese medicine compound granules + western medicine symptomatic treatment ;control group:western medicine symptomatic treatment ;

experimental group :Traditional Chinese medicine compound granules + western medicine symptomatic treatment;control group :western medicine symptomatic treatment ;

Control group:Treated according to the New Protocol for Diagnosis and Treatment of NCP(5th edition), without using traditional Chinese medicine;Experimental group 1:Basic treatment+Low dose of honeysuckle oral liquid 60ml each time, tid;Experimental group 2:Basic treatment + high dose of honeysuckle oral liquid 180 ml each time, tid;

Experimental group:Health Guidance + Guixi Regulating Lung Gong Method + Shenling Baizhu Powder + Moxibustion Treatment;Control group:Health Guidance + Guixi Regulating Lung Gong Method ;

Experimental group:ultra short wave diathermy, 10 minutes twice daily for 12 consecutive days;control:standard medical therapy;

Experimental group:atic treatment medicines;Control group:Symptomatic treatment medicines;

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Control group:Conventional treatment;Experimental group:Conventional treatment pius Dao Yin;

Experimental group:Chloroquine Phosphate;Control group:No;

experimental group.:Recombinant Human Interferon a1b Eye Drops ;Control group:placebo drop;

experimental group:mesenchymal stem cells;control group:saline;

Kesuting syrup group:conventional therapy and Kesuting syrup, p.o., 20ml, three times a day;Keqing capsule group:conventional therapy and Keqing capsule, p.o., three pills once, three times a day;control group:conventional therapy;

Chloroquine phosphate gloup:Chloroquine phosphate1.0gx2 days for the first dose, 0.5gx12 day from the third day ; Hydroxychloroquine sulfate gloup:Hydroxychloroquine sulfate 0.2g bid x 14 days;Routine treatment group:Recommended treatment plan for novel coronavirus pneumonia severe and critical cases;

group A:Arbidol Hydrochloride Tablets, Liushen Capsule, standard therapy;control group:standard therapy;

Control group:Conventional treatment;Experimental group:Conventional treatment +acupressure+Liuzijue qigong;

Experimental group:Basic treatment combined Triazavirin;Control group:Basic treatment combined Triazavirin placebo;

control group:conventional therapy;Experience group:conventional therapy and Tranilast;

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Group 1:Traditional Chinese Medicine prescription No. 1 and 2;Group 2:Gankeshuangqing capsule;Group 3:Shuanghuanglian oral liquid;

Experimental group:ozonated autohemotherapy;Control group:Standard medical treatment;

Experimental group:rhG-CSF, 5ug/kg, combinated with standard treatment;Control group:standard treatment;

Experimental group:Anti-SARS-CoV-2 virus inactivated plasma;Control group:Ordinary plasma;

1:inhalate the mycobacterium vaccae for injection;2:inhalate Physiological saline;

Experimental: Yin Hu Qing Wen Decoction Group
Based on the standard western medicine treatment, the patients will be given Yinhu Qingwen Decoction (Granula) for 10 days.
Interventions:
Drug: YinHu QingWen Decoction
Low-dose group:Tablets; 200mg; oral; twice a day; The adult dose is 1600 mg per time on first day; the duration of treatment will be 10 d.;Middle-dose group:Tablets; 200mg; orally; twice a day;The adult dose is 1800 mg per time on first day; the duration of treatment will be 10 d.;High-dose group:Tablets; 200mg; oral; twice a day; The adult dose is 2400 mg per time on first day; the duration of treatment will be 10 d.;

Experimental group:pulmonary training;control group:Conventional medication;

Experimental group:Conventional therapy and Pediatric Huatanzhike granules;Control group:Conventional therapy;

Experimental group:PD-1 mAb + Standard treatment;Control group:Standard treatment;

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Phosphoric chloroquine:2 tablets phosphoric chloroquine BID, complicated with therapy recommended by New Coronavirus Infected Pneumonia Diagnosis and Treatment Scheme (revision of Trial Version 5);Placebo:2 tablets placebo BID, complicated with therapy recommended by New Coronavirus Infected Pneumonia Diagnosis and Treatment Scheme (revision of Trial Version 5);

Intervention group 1:Oral antiviral oral solution (Xiangxue Pharmaceutical);Intervention group 2:Oral "Wu-Zhi-Fang-Guan-Fang" decoction;Control group:Routine quarantine measures;

Experimental group:TCM+Routine treatment of Western Medicine;Control group:Routine treatment of Western Medicine;

Experimental:Conventional treatment and Shenfu injection;control:Conventional treatment;

Experimental: rhACE2 group
0.4 mg/kg IV BID for 7 days (unblinded) + standard of care
Intervention: Drug: Recombinant human angiotensin-converting enzyme 2 (rhACE2)
No Intervention: Control group
Standard of care: no placebo

Hydroxychloroquine sulfate gloup:Hydroxychloroquine sulfate 0.2g bid x 14 days a day ;Chloroquine phosphate gloup:The first dose of chloroquine phosphate was 1gx2 days, and the third day was 0.5gx12 days ;Control group:Recommended treatment plan for novel coronavirus pneumonia diagnosis and treatment plan ;

Routine treatment group:Conventional treatment of 2019-nCoV pneumonia;Dipyridamole treatment group:Conventional treatment of 2019-nCoV pneumonia and Dipyridamole;

Experimental group :Oral leflunomide;Control group:Oral placebo;

experimental group:dihydroartemisinin piperaquine tablets combined with antiviral treatment.;control group:antiviral treatment was selected as: a-interferon and Ardibdol;

Psychological Intervention Group:"Intelligent Psychosomatic Regulation System" Intervention;Control Group :General Treatment ;

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Experimental group:Iv injection of Wharton's Jelly mesenchymal stem cells (1Ã—10^6/kg), cell suspension volume: 40ml;Control group:Iv 40ml saline;

Experimental group:Conventional treatent and adalimumab;Control group:conventional treatment ;

control group with common NCP:conventional treatment;experimental group with common NCP:conventional treatment combined with ozone therapy;experimental group with severe NCP:conventional treatment combined with ozone therapy;control group with severe NCP:conventional treatment;experimental group with critical NCP:conventional treatment combined with ozone therapy;control group with critical NCP:conventional treatment;

Two groups:Different stem cell doses;

Combination therapy group:Xiyanping injection+ conventional treatment;Conventional treatment group:Lopinavir / Ritonavir tablets, alpha-interferon;

Experimental group:Traditional Chinese medicine soup;Control group:Western antiviral;

Experimental group:High-dose Vitamin C;Control group:Routine treatment;

Experimental group:Intravenous injection of human umbilical cord mesenchymal stem cells (UC-MSC);Control group:Routine treatment + placebo;

Control group:Routine treatment (Pure western medicine): Lopinavir-ritonavir tablet combined with recombinant human interferon alpha 2b injection;Investigation group:(Integrated Chinese and western medicine): Lopinavir-ritonavir tablet combined with recombinant human interferon alpha 2b injection(routine treatment)+Qing-Wen Bai-Du-Yin formula granules;

Experimental Group:Conventional Treatment & Low Dose Recombinant Human Interleukin-2, i.m.(1 million IU per time, once another day, 3 times per week, 2 weeks);Control Group:Conventional Treatment+Placebo i.m.(once another day, 3 times per week, 2 weeks);

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Experimental group:umbilical cord mesenchymal stem cells;control group:convention treatment;

Experimental group:Routine treatment + plasma treatment;Control group:Routine treatment;

Experimental group:Lopinavir and Ritonavir Tablets;control group:Routine symptomatic support treatment;

Experimental group:Syndrome differential treatment of Chinese Medicine + Western medicine basic treatment;Control group:Western medicine basic treatment;

Experimental: Human Umbilical Cord-Mesenchymal Stem Cells (UC-MSCs)
Participants will receive standard of care plus 3 does of UC-MSCs
Intervention: Biological: UC-MSCs
Placebo Comparator: Placebo
Participants will receive standard of care plus 3 does of placebo
Experimental: UC-MSCs treatment group
Participants will receive conventional treatment plus 4 times of 0.5*10E6 UC-MSCs /kg body weight intravenously at Day1, Day3, Day5, Day7).
Intervention: Biological: UC-MSCs
Placebo Comparator: Control group

experimental group:Health education+ pulmonary rehabilitation;control group:Health educatio;

Experimental group 1:Routine treatment + Kanguan No. 1;Control group (Mild patients):Routine treatment;Experimental group 2:Routine treatment + Kanguan No. 2;Control group (general patient):Routine treatment;Experimental group 3:Routine treatment + Kanguan No. 3;Control group (severe patients):Routine treatment;

Experimental group of ordinary COVID-19:Lopinavir / ritonavir tablets combined with Xiyanping injection;Control group of ordinary COVID-19:Keep ritonavir/ritonavir treatment;Experimental group of severe COVID-19:Lopinavir / ritonavir tablets combined with Xiyanping injection;

1 the critical group :injecting Mesenchymal stem cells;2 the severe group:injecting Mesenchymal stem cells;3 the control group of the critical group:injecting normal saline;4 the control group of the severe group:injecting normal saline;

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Experimental group:Conventional treatment & Acupuncture;Control Group:Conventional treatment;

Experimental group:Farpiravir tablets;Control group:Abidole tablets;

Routine treatment + Jing-Yin Granule:Jing-Yin Granule 2 bags / time, 3 times a day ;Routine treatment :Treat as prescribed in the sixth edition of the diagnosis and treatment plan, without using traditional Chinese medicine;

Experimental group:Danoprevir sodium tablets,/ritonavir oral;Control group:Symptomatic treatment;

Experimental group:enteral nutrition emulsion, tpf-t 200ml Bid;Control group:No intervention;

Exocrine group:Aerosol inhalation of exosomes;Control group:Blank;

High inflammation oXiris CRRT treatment group:oXiris CRRT treatment;High inflammation Conventional treatment group:Conventional treatment;Low inflammation conventional treatment group:Conventional treatment;

experimental group:TCM formula and general treatment based on "Diagnosis and Treatment Program for COVID-19" (trial version 6th);control group:general treatment based on "Diagnosis and Treatment Program for COVID-19" (trial version 6th);

Pirfenidone group:Conventional standard treatment + Pirfenidone 400mg three times a day;Control group:Conventional standard treatment;

The experimental group:Wear a self-made "gastroscope mask" during gastroscopy;Control group:Without self-made "gastroscope mask" during gastroscopy;

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Experimental group:Acetylcysteine inhaled via tracheal tube;Control group:Placebo (saline) inhaled via tracheal tube;

Control group:conventional therapy;CIK group:Umbilical cord blood CIK cells (1.6Ã—10E8/kg) were injected twice every other day.;NK group:Umbilical cord blood NK cells (1.6Ã—10E8/kg) were injected twice every other day.;

Experimental: DAS181 SD group Cohort 1, Stage 1
DAS181 SD group 4.5mg/day for 7 or 10 days
Intervention: Drug: DAS181
Experimental: DAS181 HD group Cohort 1, Stage 1
DAS181 HD group 9mg/day for 7 or 10 days

Experimental group:Conventional treatment and anti-SARS-CoV-2 virus inactivated plasma;Control group:Conventional treatment and Ordinary plasma;

Experimental group:On the basis of routine treatment, add 100ml of normal saline and 50ml of Xuebijing injection twice a day;;Control group:conventional therapy;

Experimental group:Capsules of bismuth potassium citrate, 2 capsules at a time, once every 12 hours; take warm water with breakfast and dinner; do not take high protein foods such as milk at the same time.;Control group:Simulated capsules of bismuth potassium citrate, 2 capsules at a time, once every 12 hours; take warm water with breakfast and dinner; do not take high protein foods such as milk at the same time.;

Experimental group:Combined standard therapy of chloroquine phosphate aerosol inhalation solution;control group:Water for injection atomization inhalation combined with standard therapy;

Group 1:Eight lung exercises for rehabilitation;Group 2:Free activities;

Control group:Conventional integrated traditional Chinese and western medicine treatment and western medicine psychological intervention treatment;Experimental group:Conventional integrated traditional Chinese and western medicine treatment, western medicine psychological intervention therapy and traditional chinese medicine psychological intervention therapy;

Case series:rehabilitation and Lung eight-segment exercise;Control group:Keep the initial life style;

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Group 2:SMO Assisted tracheal intubation;Group 1:Assisted tracheal intubation via nasal high-flow oxygen inhalation;

Control group:TCM + Conventional medicine treatment + conventional medicine's psychological intervention;Experimental group:Integrated Chinese and Conventional medicine treatment + conventional medicine's psychological intervention + TCM psychological intervention;

Control group:Coventional Treatment;Experimental group:Conventional treatment+LIUSHENWAN;

Lipoic acid group:Routine Therapy and administration of Lipoic acid injection;Blank control group:Routine Therapy and administration of placebo;

1:meditation+Standard treatment of traditional Chinese and Western Medicine;2:Standard treatment of traditional Chinese and Western Medicine;

Positive drug control group:Refer to the National Health Office Medical Letter (2020) 77 document "Diagnosis and treatment plan for the novel coronavirus disease (trial version 5)" general treatment plan. Based on the general treatment plan of Western medicine + Lianhua Qingwen Capsule (0.35 g per capsule) orally, 4 capsules each time;Compound Yuxingcao Mixture Group:Refer to the National Health Office Medical Letter (2020) 77 Control group:Refer to the National Health Office Medical Letter (2020) 77 document "Diagnosis and treatment plan for the novel coronavirus disease (trial version 5)" general treatment plan.;Experimental group:Refer to the National Health Office Medical Letter (2020) 77 document "Diagnosis and treatment plan for the novel coronavirus disease (trial version 5)" general treatment plan. Based on the general treatment plan of western

Interferon Therapy Group:Standard treatment + cerrokin (recombinant human interferon alpha 1beta) 10UG BID is administered by nebulization for 10 days.;Control group:Standard treatment + blank;

Early corticosteroid intervention group:Early corticosteroid intervention;Middle-late corticosteroid intervention group:Middle-late corticosteroid intervention;Control group:No corticosteroid;

treatment group :Ba duanjin;

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treatment group1:HUMSCs: intravenous infusion, 5 Ã— 10E7 cells / time, once / week, twice / course;treatment group 2:HUMSCs: intravenous infusion, 5 Ã— 107 cells / time, 1 time / week, 2 times / course, a total of 2 courses; Exosomes: intravenous administration, 180mg / time, 1 time / day, 7 days / course, 2 courses in total;control group:The control group was given the same amount of placebo (stem cell solvent);

Experimental: Tetrandrine Cohort
After the subjects were enrolled, they were given "Tetrandrine 60mg QD" for a course of 1 week(Take 6 days, stop using for 1 day)
Intervention: Drug: Tetrandrine
No Intervention: Control Cohort

experimental group:Intravenous infusion of NK cells on day 0,3 and 6 , 1*107â€”1*108/kg body weight of NK cells each day.;control group:Sodium Potassium Magnesium Calcium and Glucose Injection.;

Experimental group:Intravenous drip, 0.2 g (4 vials) once daily for adults; slow drip after dilution with 250 ml of 5% Glucose Injection or 0.9% Sodium Chloride Injection, not less than 120 minutes or 30-40 drops/minute. Continuous administration for 7 days, observation until 21 days or discharge;Control group:Conventional treatment, no control drug;

experimental group:Ma-Xing-Gan-Shi decoction and Conventional Western Medicine Treatment;control group:placebo and Conventional Western Medicine Treatment;

experimental group:ebastine 10mg bid, nterferon-alpha aerosol inhalation 5million U bid and Lopinavir 200 mg, 2 capsules at a time, bid;control group:interferon-alpha aerosol inhalation 5million U bid and Lopinavir 200 mg, 2 capsules at a time, bid;

CRRT group:CRRT;Control group:Routine treatment, CRRT only when there are emergency indications;

Control group:Basic treatment refers to the treatment according to the diagnosis and treatment plan of the health care Commission (Sixth Edition). (after three days of medication, if the symptoms cannot be controlled in time and effectively, the patients can be sent out of the group immediately to receive the treatment&#;Experimental group 1:Basic treatment + honeysuckle oral liquid at a low dose of 60ml / time, three times a

Experimental group:Lung rehabilitation training+Conventional treatment;Control group:Conventional treatment;

experimental group:Based on the standard treatment recommended in the guidelines, a combination of tozumab and adamumab was used;control group:Follow the guidelines for standard treatment;

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Experimental group:Convalescent plasma therapy + routine treatment;Control group:routine treatment;

Experimental: Favipiravir Combined With Tocilizumab group
Favipiravir: On the 1st day, 1600mg each time, twice a day; from the 2nd to the 7th day, 600mg each time, twice a day. Oral administration, the maximum number of days taken is not more than 7 days.

Group A:Sevoflurane anesthesia maintenance, common extubation;Group B:Sevoflurane anesthesia was maintained, and the extubation strategy of "circulatory induced respiration" was adopted after the operation.;Group C:Propofol anesthesia maintenance, common extubation;Group D:Propofol anesthesia was maintained, and the extubation strategy of "circulatory induced respiration" was adopted after the operation.;

Experimental: Bevacizumab Group
Intervention: Drug: Bevacizumab
No Intervention: Control Group

experimental group:Based on the standard treatment recommended in the guidelines, a combination of Prolongin (Enoxaparin Sodium Injection) was used.;control group:Follow the guidelines for standard treatment.;

experimental group:Based on the standard treatment recommended in the guidelines, a combination of Prolongin (Enoxaparin Sodium Injection) was used.;control group:Follow the guidelines for standard treatment.;

eperimental group:Conventional treatment and convalescent plasma therapy;control group:conventional treatment;

experimental group:Ixekizumab and antiviral therapy;control group:antiviral therapy;

Experimental group:Bufonis venenum injection;Control group:0.9% N.S.;

Experimental group:chloroquine phosphate ;Control group:none;

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Group 2:Western medicine conventional treatment;Group 1:TCM treatment;

common type :Immunotherapy with recombinant chimeric DC vaccine;Latent period group:Immunotherapy with recombinant chimeric DC vaccine;Normal control group:Immunotherapy with recombinant chimeric DC vaccine;Blank vaccine control group:0.9%NS;

control group:Routine supportive treatment;experimental group:TCM decoction intervention + routine supportive treatment;

Experimental group:Conventional treatent and Ulinastatin;Control group:Conventional treatent ;

Exocarpium Citri Grandis Group:Oral Administration of Exocarpium Citri Grandis Phlegm Cough Solution;Placebo Group:Oral Administration of Placebo;

Experimental: Yinhu Qingwen Granula Group
Based on the standard medical treatment, the patients will be given Yinhu Qingwen Granula for 10 days.
Interventions:
Drug: Yinhu Qingwen Granula

Experimental group:Pirfenidone;Control group:Blank;

Control group:Routine medicines and food;Experimental group:Taking Newgen beta-gluten probiotic composite powder;

Tricholoma matsutake group:Taking tricholoma matsutake ;Cannabis sativa capsule group:Taking cannabis sativa capsule ;Dendrobium candidum group:Taking dendrobium candidum;Routine treatment group:Taking routine drugs and food;

A:Long-acting interferon alpha-2a (135ug) + ribavirin;B:Abidol + ribavirin;

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Suspected case treatment group:TCM formula 1 or TCM formula 2;Suspected case control group:null;Confirmed case treatment group:Western medicine+(TCM formula 3 or TCM formula 4 or TCM formula 5 or TCM formula 6);Confirmed case control group:Western medicine;

Experimental group:Anti-SARS-CoV-2 virus inactivated plasma;Control group:Ordinary plasma;

Group 1:Liuzijue exercise;Group 2:Respiratory muscle training;Group 3:Liuzijue exercise combined with respiratory muscle training;

Experimental group:On the basis of rehabilitation treatment, the subjects in the experimental group take Gu-Shen Ding-Chuan-Wan;Control group:standard treatment;

Experimental group:On the basis of the current clinical treatment of SNCP, NK cells and MSCs were increased;Control group:current clinical treatment of SNCP;

Experimental: favipiravir tablets+chloroquine phosphatetablets tablets group
favipiravir tablets+chloroquine phosphatetablets tablets
Intervention: Drug: favipiravir tablets+chloroquine phosphatetablets tablets
Experimental: favipiravir tablets group
favipiravir tablets

Experimental group:Hua-Shi Bai-Du granules;control group:conventional treatment of western medicine;

Experimental group:normal treatment and auricular point pressing;control group:normal treatment;

Group 1:psychological support;Group 2:cognitive behavioral therapy;

Pirfenidone group:Support treatment with oral taken Pirfenidone capsule;Support treatment group:Support treatment ;

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Experimental group vs control group:Hydroxychloroquine versus Placebo;

Experimental: Bone Marrow-Derived Mesenchymal Stem Cells (BM-MSCs)
Conventional treatment plus BM-MSCs
Intervention: Biological: BM-MSCs
Placebo Comparator: Placebo
Conventional treatment plus placebo

Experimental treatment group:Oral chloroquine phosphate tablets (treatment group);control group:Oral placebo group (control group);

Experimental: oxyhydrogen
conventional treatment + hydrogen/ oxygen inhaled
Intervention: Device: oxyhydrogen
Experimental: oxygen
conventional treatment + oxygen inhaled

Experimental: NK cells
The NK cells are going to be give by intravenous infusion (10E8 cells per kilogram of body weight, once a week) .
Intervention: Biological: NK cells,IL15-NK cells,NKG2D CAR-NK cells,ACE2 CAR-NK cells,NKG2D-ACE2 CAR-NK cells

Active Comparator: Control group
conventional symptomatic treatments such as antiviral (oseltamivir), hormones, oxygen therapy, mechanical ventilation and other supportive therapies
Interventions:
Drug: Oseltamivir

Control group:regular treatment;Experimental group:Compound amino acid combined with routine treatment;

Group M:Play sleep-aid music on the cell phone at the same time as basic therapy, listen to it once every night at 22 o'clock, and the patient falls asleep with the music. A total of 2 weeks of treatment.;Group C:No music, the use of basic treatment, in accordance with the "new coronavirus infection pneumonia diagnosis and treatment program" trial fifth, sixth, seventh version of the provisions of the implementation ;

Experimental: hDPSCs group
Routine treatment + Intravenous injection of human dental pulp stem cells
Intervention: Biological: allogeneic human dental pulp stem cells (BSH BTC & Utooth BTC)
Placebo Comparator: Control group
Routine treatment + Intravenous saline injection (Placebo)

Nintedanib group:Nintedanib;Control group:Placebo;

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experimental group:Rabeprazole plus chloroquine;control group :Lopinavir + rabeprazole;

Experimental: Favipiravir group
On the 1st day, 1600mg each time, twice a day; from the 2nd to the 7th day, 600mg each time, twice a day. Oral administration, the maximum number of days taken is not more than 14 days.
Intervention: Drug: Favipiravir

Experimental: Anluohuaxian combined with regular treatment group
Anluohuaxian: 6g each time, twice a day
Intervention: Drug: Anluohuaxian
No Intervention: regular treatment group

Control:NA;Noninvasive cardiopulmonary monitoring group:Wear non-invasive multi-channel cardiopulmonary monitor;

Experimental Group:Xiaotan Sanjiefang granule;Control Group:placebo (contain 5% test drug);

Active Comparator: Arm 1
1×10^11vp of Ad5-nCoV administered through 1.0 mL intramuscular injection in the deltoid muscle on Day 0
Intervention: Biological: Recombinant novel coronavirus vaccine (Adenovirus type 5 vector)
Active Comparator: Arm 2

NK cell group:NK cells were injected by intravenous drip once a day for 2 to 3 times in total, each dose was 4 10^7 pieces /kg body weight,100ml normal saline suspension;Control group:100ml normal saline intravenous drip;

Phase I A1:Low dose;Phase I A2:Placebo;Phase I A3:Middle dose;Phase I A4:Placebo;Phase I A5:High dose;Phase I A6:Placebo;Phase I E1:Low dose;Phase I E2:Placebo;Phase I E3:Middle dose;Phase I E4:Placebo;Phase I E5:High dose;Phase I E6:Placebo;Phase I F1:Low dose;Phase I F2Placebo;Phase I F3:Middle dose;Phase I F4:Placebo;Phase I F5:High dose;Phase I F6:Placebo;phase II A1:Low dose;phase II

Experimental group:exercise rehabilitation under remote monitoring;Control group:patient education;

experimental group:"Guangdong Pneumonia NO.1"+basic treatment; control group:basic treatment and(or) antiviral therapy;

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Experimental: Emergency schedule & Two doses of medium dosage vaccine
24 participants in phase Ⅰand 60 participants in phase Ⅱ will receive two doses of medium dosage inactivated SARS-CoV-2 vaccine at the emergency vaccination schedule,and 60 participants in phase Ⅱ will receive one dose of booster immunization with medium dosage vaccine 6 months after the emergency schedule

experimental group:Chinese herbal tea;control group:drinking water;

control group:General basic epidemic prevention measures;sachet group:General basic epidemic prevention measures + Traditional Chinese medicine sachet;Oral and nasal spray group:General basic epidemic prevention measures + Oral and nasal spray;Oral Chinese medicine group:General basic epidemic prevention measures + Traditional Chinese medicine for epidemic prevention in high risk groups;

1.5ATA:1.5ATA HBO;2.5ATA:2.5ATA HBO;Control group:Routine treatment;

Experimental group: mild cases:Ulinastatin 300,000 IU, Q12H;Experimental group: severe or critical cases:Severe cases: 1 million IU of ulinastatin, Q8H; Critical cases: ulinastatin 1.6 million IU, Q8H;Control group: mild cases:conventional therapy;Control group: severe or critical cases :conventional therapy;

Experimental: Moxibustion plus Cupping
Moxibustion plus cupping and basic therapy (rehabilitation direction and basic breathing exercise) once a day for 4 weeks (28 sessions).The moxibustion plus cupping treatments were divided into 2 alternating formulas:

Control Group:Qimaifeiluoping Mixture Simulator + Respiratory Training + Rehabilitation Education Guidance;Experimental group:Qimaifeiluoping Mixture + Respiratory Training + Rehabilitation Education Guidance;

Experimental group:Shengmai Decoction;Control group:Shengmai Decoction simulant;

Experimental group:Shumian capsule;Control group:Shumian capsule placebo;

Experimental group:Basic treatment + kolimycin;Control group:Basic treatment;

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No intervention: the NCP standard treatment group
According to the diagnosis and treatment guidelines, the patients were divided into four types: mild, common, severe and critically ill. Then patients with common and severe ill were randomly divided into two groups, respectively, namely the NCP standard treatment group and the NCP aspirin group (aspirin 100 mg/d, oral + combined standard treatment)

Experimental group:Jiexingjunzi granules ;control group:placebo;

Group 2:Aliskiren with Standard Care;Group 1:Nifedipine with Standard Care;

Group 2:Conventional treatment;Group 2:Conventional treatment plus liu-zi--jue exercisee therapy;

Control group:xiaoyao capsule;Experimental group:xiaoyao capsule;

experimental group:Perform respiratory rehabilitation exercises.;control group:No respiratory rehabilitation exercise;

Phase I A1:Low dosage;Phase I A2:Placebo;Phase I A3:medium dosage;Phase I A4:Placebo;Phase I A5:high dosage;Phase I A6:Placebo;Phase I F1:Low dosage;Phase I F2:Placebo;Phase I F3:medium dosage;Phase I F4:Placebo;Phase I F5:high dosage;Phase I F6:Placebo;Phase I G1:low dosage;Phase I G2:placebo;Phase I G3:medium dosage;Phase I G4:placebo;Phase I G5:high dosage;Phase I

Experimental group:Administration of hydroxychloroquine sulfate;control group:Placebo given;

control group:conventional therapy;Experimental group:Bufei Huoxue Capsule, 1.4g/day, three times a day, three months of treatment;

Control group:the basic treatment withThymosin enteric capsule + Pien Tze Huang Capsules placebo;Experimental group:the basic treatment with Thymosin enteric capsule +Pien Tze Huang Capsules;

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Routine treatment group:Routine treatment;WMT group:Washed microbiota transplantation;

Experimental Group:FNC+Standard of Care;Control Group:FNC dummy tablet+ Standard of Care;

Experimental: Experimental Vaccine-medium dosage
24 participants at medium dosage stage in phase I will receive two doses of primary immunization according to the immunization schedule of day 0, 28 and will receive 1 dose of booster immunization 1 year after primary immunization; 100 participants at medium dosage stage in phase II will receive two doses of primary immunization according to the

Experimental: berberine group (B group)
Patients in the B group were given berberine hydrochloride tables 0.3g tid orally or tube feed daily, until the 14th day of the study. Other treatments include general support therapy, oxygen therapy, antiviral drugs, in combination with antibiotics and small doses of glucocorticoids if necessary, nutritional and organ function support

Experimental group:Taking artemisinin-piperaquine;Control group:Symptomatic treatment with non-antiviral drugs;

Experimental group:Lu-Dang-Shen oral solution 10ml / time, 2 times a day;Control group:Placebo 10ml / time, 2 times a day;

Group A:self-service pulmonary rehabilitation;Group B:Internet-assisted pulmonary rehabilitation;

Experimental group:standard treatment + shuanghuanglian treatment;Control group:standard treatment;

Experimental: Low dosage vaccine on a 0- and 28-day schedule
Two doses of low dosage Inactivated SARS-CoV-2 Vaccine at the vaccination schedule of day 0, 28
Intervention: Biological: Low dosage Inactivated SARS-CoV-2 Vaccine on a 0- and 28-day schedule

intervention group:abdominal massage + ST-36 (Zusanli) acupressure;control group:regular nursing;

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Experimental group:Western medicine(pirfenidone)+chinese herb;control:Pirfenidone drugs + traPirfenidone + placebo of traditional Chinese Medicine;

Experimental Group:Baicoli chewable tablets, 200mg / tablet, dosage 200mg / Time, 3 times a day, total 600mg / day, total course of treatment: 10 days;Control Group:Placebo 200 mg / day, 3 times a day, 600 mg / day, for 10 days;

Experimental group:Conventional treatment plus Liu--zi-jue exercise therapy;Control group:Conventional treatment;

Control:Guidelines for routine treatment;Therapy group:TCM;

Experimental: Test group
Intervention: Combination Product: JS016 (anti-SARS-CoV-2 monoclonal antibody)
Experimental: Control group
Intervention: Combination Product: JS016 (anti-SARS-CoV-2 monoclonal antibody)

Experimental group:Bromhexine Hydrochloride Tablets + Standard treatment;Control group:Standard treatment;

Experimental: Population I
Population I has 20 subjects and is considered negative for SARS-COV-2 and fluorescent RT-PCR nucleic acids. Population I is intramuscular injection of deltoid muscle of upper arm with low dose of vaccine.
Intervention: Biological: Recombinant new coronavirus vaccine (CHO cell) low-dose group

Ninety-three eligible participants selected by cluster sampling are randomized to an intervention group (n = 47) and a control group (n = 46) using a computerized random number generator by a trial statistician who had no clinical involvement in the project. Participants in the control group receive routine treatment according to the Chinese Management Guidelines for COVID-19, while participants in the intervention group receive

Low dose group:Investigational vaccine or placebo of corresponding dose;Medium dose group:Investigational vaccine or placebo of corresponding dose;High dose group:Investigational vaccine or placebo of corresponding dose;

Experimental: Population I
In population I, there were 150 subjects who injected with 2 doses of low-dose test vaccine into the deltoid muscle of the upper arm according to the 0 and 1 month immunization schedule.
Intervention: Biological: Recombinant new coronavirus vaccine (CHO cell) low-dose group

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Experimental: Low dosage vaccine on a 0- and 28-day schedule
Two doses of low dosage Inactivated SARS-CoV-2 Vaccine at the vaccination schedule of day 0, 28
Intervention: Biological: Low dosage Inactivated SARS-CoV-2 Vaccine on a 0- and 28-day schedule

Experimental: Conort 1
BRII-196 dose level 1 or placebo
Interventions:
Drug: BRII-196
Drug: Placebo

Experimental: Conort 1
BRII-198 dose level 1 or placebo
Interventions:
Drug: BRII-198
Drug: Placebo

Experimental: anti-SARS-CoV-2 monoclonal antibody(SCTA01)
SCTA01: single dose on Day0
Intervention: Biological: SCTA01
Placebo Comparator: Placebo
Placebo: single dose on Day0

Experimental group:Sancai granule;Control group:stimulant;

Group 1:Atomization treatment, 0.2mg IFN-k/ 0.5 mg TFF2;Group 2:Atomization treatment, 1mg IFN-k/ 2.5 mg TFF2;Group 3:Atomization treatment, 2mg IFN-?/ 5 mg TFF2;Group 4:Atomization treatment, 4mg IFN-?/ 10 mg TFF2;

Experimental: Population I
Population I has 20 subjects and is considered negative for SARS-COV-2 and fluorescent RT-PCR nucleic acids. Population I is intramuscular injection of deltoid muscle of upper arm with low dose of vaccine.
Intervention: Biological: Biological/Vaccine: Recombinant new coronavirus vaccine (CHO

Experimental: Low-dose, 18-55 years of age
Intervention: Biological: BNT162b1
Experimental: High-dose, 18-55 years of age
Intervention: Biological: BNT162b1
Placebo Comparator: Placebo, 18-55 years of age

Experimental: MW33
Intervention: Combination Product: MW33 injection
Experimental: Placebo
Intervention: Combination Product: MW33 injection placebo

Experimental: Middle-dose vaccine (18-55 years) & Two dose regimen
Two doses of middle-dose experimental vaccine at the schedule of day 0, 28.
Intervention: Biological: Two doses of middle-dose recombinant SARS-CoV-2 vaccine (Sf9 Cell) at the schedule of day 0, 28
Experimental: Middle-dose vaccine (≥ 55 years) & Two dose regimen

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Intervention group:Mindfulness-based interventions (MBIs) ;Control group:Health education;

Experimental group:vaccination;Placebo control group:vaccination;

experimental group:Vaccine INO-4800;control group:placebo;

Experimental: Experimental Vaccine-low dosage
low dosage inactivated SARS-CoV-2 vaccine
Intervention: Biological: Two doses of low dosage inactivated SARS-CoV-2 vaccine at the schedule of day 0,28
Experimental: Experimental Vaccine-medium dosage

Jianpi Kanggu Group:Jianpi Kanggu Granules;Jintiange Group:Jintiange Capsule;

Jianpi Kanggu Granules Group:Jianpi Kanggu Granules;Xianling Gubao Capsule Group:Xianling Gubao Capsule;

Experimental: MID A
20 participants(18-49), Ad5-nCoV , two doses, Intramuscular administration
Intervention: Biological: Recombinant Novel Coronavirus Vaccine (Adenovirus Type 5 Vector)
Placebo Comparator: MID B
Active Comparator: Anti-SARS-CoV-2 Monoclonal Antibody dose 1/2/3
use Anti-SARS-CoV-2 Monoclonal Antibody,dose 1/2/3 to treat COVID-19
Intervention: Biological: Recombinant Human Anti-SARS-CoV-2 Monoclonal Antibody(25mg/kg;50mg/kg;100mg/kg)
Placebo Comparator: Placebo

Jiexingjunzi granule group:Jiexingjunzi granules;Qibei Fuzheng granule formation:Qibei Fuzheng granule;

A1: 3 doses, medium dosage or placebo; A2: 3 doses, high dosage or placebo; A3: 2 doses, medium dosage/placebo; A4: 2 doses, high dosage/placebo; B1: medium dosage/placebo; B2: high dosage/placebo

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Experimental: experimental Group
the experimental arm will receive hydrogen-oxygen mixed gas inhalation (Hydrogen-Oxygen Generator with Nebulizer, AMS-H-03, output: 3 L/min (hydrogen concentration: 66.7%, oxygen concentration: 33.3%)) ,the treatment duration will be 8 hours per day, for 12 weeks

A:Vaccinated with 5ug vaccine or placebo;B:Vaccinated with 10ug vaccine or placebo;C:Vaccinated with 15ug vaccine or placebo;D:Vaccinated with 20ug vaccine or placebo;E:Vaccinated with 25ug vaccine or placebo;

experimental group:Personalized psychological intervention plus narrative exposure therapy;control group:Personalized psychological intervention;

A1:medium dosage;A2:high dosage;A3:placebo;A4:medium dosage;A5:high dosage;A6:placebo;A7:medium dosage;A8:high dosage;A9:placebo;B1:medium dosage;B2:high dosage;B3:placebo;

Experimental: Aged 26-45, Commercial Scale
Commercial scale inactivated SARS-CoV-2 vaccine in adults aged 26-45 years.
Intervention: Biological: Two doses of commercial scale inactivated SARS-CoV-2 vaccine at the schedule of day 0,14
Experimental: Aged 18-59, Pilot Scale

Experimental group:0,14 day immunization procedure;Control group:0,14 day placebo of immunization procedure;Experimental group:0, 21 day immunization procedure;Controll group (0,21 day immunization procedure):0,21 day placebo of immunization procedure;

Active Comparator: MW33 injection-1200mg
Intervention: Combination Product: MW33 injection
Active Comparator: MW33 injection-2400mg
Intervention: Combination Product: MW33 injection
Placebo Comparator: Placebo
Experimental: BNT162b2 18-85 years of age
Intervention: Biological: BNT162b2
Placebo Comparator: Placebo 18-85 years of age
Intervention: Other: Placebo
Experimental: Middle-dose vaccine (18-59 years) & Two dose regimen
Two doses of middle-dose experimental vaccine at the schedule of day 0, 14.
Intervention: Biological: Two doses of middle-dose recombinant SARS-CoV-2 vaccine (CHO Cell) at the schedule of day 0, 14
Experimental: Middle-dose vaccine (18-59 years) & Three dose regimen
Experimental: Low-dose vaccine (18-59 years) & Two dose regimen
two doses of low-dose Recombinant COVID-19 vaccine (Sf9 cells) at the schedule of day 0, 21.
Intervention: Biological: Low-dose Recombinant COVID-19 vaccine (Sf9 cells) (18-59 years) & Two dose regimen

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A1 Adult group:INO-4800 1.0 mg / placebo;A2 Elderly group:INO-4800 1.0 mg / placebo;B1 Adult group:INO-4800 2.0 mg/ placebo;B2 Elderly group:INO-4800 2.0 mg/ placebo;

Two cohorts:Test vaccine and placebo control;

Experimental: moderate COVID-19 group
patients with moderate COVID-19 receiving western medicine treatment
Intervention: Drug: Alpha-interferon alpha, abidol, ribavirin, Buzhong Yiqi plus and minus formula, Huhuang Detoxicity Paste, Baimu Qingre Jiedu Paste, fumigation/inhalation of vitamin C

Control Group:Standard care;Aerosol inhalation:Standard care plus aerosol inhalation;

Experimental: Cohort 1
BRII-196 and BRII-198 dose level 1 or placebo
Interventions:
Drug: BRII-196 and BRII-198
Drug: Placebo

0, 14 emergency vaccination group:0, 14 day immunization procedure;0, 21 emergency vaccination group:0, 21 day immunization procedure;0, 28 emergency vaccination group:0, 28 day immunization procedure;

0, 14 emergency vaccination group:two doses of vaccine at the schedule of day 0, 14;0, 21 emergency vaccination group:two doses of vaccine at the schedule of day 0, 21;0, 28 emergency vaccination group:two doses of vaccine at the schedule of day 0, 28;

0,14 emergency vaccination group:two doses of vaccine at the schedule of day 0,14;0, 21 emergency vaccination group:two doses of vaccine at the schedule of day 0, 21;0,28 emergency vaccination group:two doses of vaccine at the schedule of day 0, 28;

A:COVID-19 mRNA vaccine 5ug/0.5ml;B:COVID-19 mRNA vaccine 10ug/0.5ml;C:COVID-19 mRNA vaccine 15ug/0.5ml;D:Placebo;

Experimental group:Routine treatment + Lian-Hua Qing-Ke Tablets;Control group:Conventional treatment;

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Experimental group:Routine treatment + Lian-Hua Qing-Ke Tablets;Control group:Conventional treatment;

Experimental group:Lian-Hua Qing-Wen Capsule/Granule;Control group:Isolated observation;

JMB2002 5mg/Kg group:5mg/Kg dose of JMB2002 single intravenous infusion;JMB2002 15mg/Kg group:15mg/Kg dose of JMB2002 single intravenous infusion;JMB2002 15mg/Kg group:30mg/Kg dose of JMB2002 single intravenous infusion;JMB2002 50mg/Kg group:50mg/Kg dose of JMB2002 single intravenous infusion;

Experimental: adults group (aged 18-59 years) & vaccine
three doses of 40μg Recombinant COVID-19 vaccine (Sf9 cells) at the schedule of day 0, 21, 42.
Intervention: Biological: Recombinant COVID-19 vaccine (Sf9 cells)
Placebo Comparator: adults group (aged 18-59 years) & placebo

Experimental group: Qing-Jin Yi-Qi granule + rehabilitation treatments; Control group: rehabilitaion treatments;

Lian-Hua Qing-Wen Capsule Group:4 capsules/time, 3 times a day;Isolated observation control group:In accordance with the management requirements of the "New Coronavirus Pneumonia Prevention and Control Plan (Seventh Edition)" issued by the State Council, a 14-day quarantine medical observation was given;

Experimental group:Shugan Jieyu Capsules;Control group:Shugan Jieyu Capsule Simulator;

Experimental: BRII-196 and BRII-198 in adult subjects with severe COVID-19
Intervention: Drug: BRII-196 and BRII-198
Experimental: BRII-196 and BRII-198 in adult subjects with mild-moderate COVID-19
Intervention: Drug: BRII-196 and BRII-198
Experimental: Placebo in adult subjects with mild-moderate COVID-19

inactivated vaccine group:Novel Coronavirus inactivated vaccine (Vero cell);control group:Nil;

Experimental: Combined immunization group
The combined immunization group is randomly divided into two subgroups, 120 subjects in each group. The combined immunization subgroup I receive SARS-CoV-2 inactivated vaccine (Vero cell)&Quadrivalent Influenza Vaccine on day0 and SARS-CoV-2 inactivated vaccine (Vero cell) (second dose) on day 28 The combined immunization subgroup II

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Experimental: Middle-dose vaccine (18-59 years)
Three doses of middle-dose experimental vaccine at the schedule of day 0, 28, 56
Intervention: Biological: a middle-dose recombinant COVID-19 vaccine (CHO Cell) (18-59 years) at the schedule of day 0, 28, 56
Experimental: High-dose vaccine (18-59 years)

Trial group:Qingfei Paidu Granule;Control group: Conventional treatment;

Experimental: "0-28 days" vaccine group
One dose of recombinant Ad5 vectored COVID-19 vaccine on day 0, the second dose of subunit vaccine (ZF2001) against COVID-19 on day 28, and a third of subunit vaccine (ZF2001) against COVID-19 on month 4.
Interventions:
Group X: Two-dose 10-ug youth (18-59 years) group and two-dose 25-ug youth (18-59 years) group:All the subjects are administrated V-01 or placebo on Days 0 and Day 21 at the deltoid muscle in the upper arm and tested for safety, tolerance and immunogenicity as per the schedule described in the study protocol.;Group Y: Two-dose 10-ug elder (>=60 years) group and two-dose 25-ug elder (>=60 years) group:All the subjects are
Group A: 10-ug youth (18-59 years) group:All subjects are given one dose of V-01 or placebo on Days 0 and 21.;Group B: 25-ug youth (18-59 years) group:All subjects are given one dose of V-01 or placebo on Days 0 and 21.;Group C: 50-ug youth (18-59 years) group:All subjects are given one dose of V-01 or placebo on Days 0 and 21.;Group D: 10-ug elder (>=60 years) group:All subjects are given one dose of V-01 or placebo on Days 0

Intervention group:vaccinated in the morning;Control group:vaccinated in the afternoon;

Experimental: A1a Phase I low 2 doses
Ad5-nCoV containing 0.5E10 vp, 2 doses 56 days apart, Aerogen Solo
Intervention: Biological: Recombinant Novel Coronavirus Vaccine (Adenovirus Type 5 Vector) for Inhalation
Placebo Comparator: A1b Phase I placebo low 2 doses
Experimental: Phase 1 low-dose group
Intervention: Biological: low-dose Recombinant SARS-CoV-2 Vaccine (CHO cell)
Experimental: Phase 1 high-dose group
Intervention: Biological: high-dose Recombinant SARS-CoV-2 Vaccine (CHO cell)
Placebo Comparator: Phase 1 placebo group
Experimental: A1 aged ≥ 71
300 subjects age ≥ 71 （A1）receive 3 doses of vaccine
Intervention: Biological: 3-doses schedule 1 of COVID-19 Vaccine (Vero Cell), Inactivated
Experimental: A2 aged ≥ 71
200 subjects are ≥ 71 （A2）receive 3 doses of vaccine
Low dose adult group: mRNA COVID-19 vaccine/Placebo; Medium-dose adult group: mRNA COVID-19 vaccine/Placebo; Secondary high-dose adult group: mRNA COVID-19 vaccine/Placebo; High dose adult group: mRNA COVID-19 vaccine/Placebo; Low dose older group: mRNA COVID-19 vaccine/Placebo; Medium-dose older group: mRNA COVID-19 vaccine/Placebo; Secondary high-dose older group: mRNA COVID-19 vaccine/Placebo;

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Experimental: Experimental Group
Participant will receive two doses of medium dosage inactivated SARS-CoV-2 vaccine . Vaccine will given by intramuscular injection on day 0 and day 28.
Intervention: Biological: SARS-CoV-2 Inactivated Vaccine
Placebo Comparator: Control Group

Experimental group: 1st dose : combined vaccination of COVIV+IIV4; 2nd dose: combined vaccination of COVIV+PPV23; Control group A: 1st dose: COVIV only; 2nd dose: COVIV only; Control group B: 1st dose: IIV4 only; 2nd dose: PPV23 only;

Experimental group: 1st dose : combined vaccination of COVIV+PPV23 ; 2nd dose: combined vaccination of COVIV+IIV4; Control group 1: 1st dose: COVIV only; 2nd dose: COVIV only; Control group 2: 1st dose: PPV23 only; 2nd dose: IIV4 only;

treatment group:Qingjin Yiqi granule + comprehensive rehabilitation therapy of traditional Chinese and Western Medicine;Control group:comprehensive rehabilitation therapy of traditional Chinese and Western Medicine;

Experimental: nicotinamide plus usual care
Intervention: Drug: nicotinamide
No Intervention: usual care

Biological: Recombinant Novel Coronavirus Vaccine (Adenovirus Type 5 Vector) 0.5ml;Biological: Recombinant Novel Coronavirus Vaccine (Adenovirus Type 5 Vector) 0.3ml

Experimental: heterologous boost arm with Ad5 vectored vaccine
Subjects who have been primed with two doses of inactive SARS-CoV-2 vaccine will receive a booster of recombinant SARS-CoV-2 Ad5 vectored vaccine after 3~6 months.
Intervention: Biological: Recombinant SARS-CoV-2 Ad5 vectored vaccine
Active Comparator: homogeneous boost arm with inactive vaccine

Experimental group:DelNS1-2019-nCoV-RBD-OPT1;Control group:Placebo;

inactivated vaccine group:inactivated Novel Coronavirus Pneumonia vaccine (Vero cells);mRNA vaccine group:Novel Coronavirus Pneumonia mRNA vaccine (DF104B1);

Treatment Group of Xinguan Fuzheng Zhuanyin Decoction :Western medicine standard treatment combined with Xinguan Fuzheng Zhuanyin Decoction;Placebo group:Western medicine standard treatment;

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Experimental: SCB-2019 vaccine
Investigational SCB-2019 vaccine contains 30 μg of SCB-2019 antigen, and 1.5 mg CpG 1018 and 0.75 mg Alhydrogel as adjuvants, in each 0.5 mL dose
Intervention: Biological: Candidate Vaccine, SCB-2019
Placebo Comparator: Placebo
Experimental: 30mg LY-CovMab
LY-CovMab is diluted with 0.9% sodium chloride to a total volume of 250 mL for IV infusion
Intervention: Drug: LY-CovMab
Experimental: 150mg LY-CovMab
LY-CovMab is diluted with 0.9% sodium chloride to a total volume of 250 mL for IV infusion
Experimental: Experimental Group
180 subjects (including 90 adults aged 18-59 years and 90 elderly aged 60 year and older) will receive two doses of inactivated COVID-19 vaccine on day 0 and day 28
Intervention: Biological: Inactivated COVID-19 Vaccine
Placebo Comparator: Control Group
Experimental: Population I
Population I has 20 subjects aged 12 to 17 and is considered negative for SARS-COV-2 and fluorescent RT-PCR nucleic acids. PopulationIis intramuscular injection of deltoid muscle of upper arm with vaccine.
Intervention: Biological: Recombinant new coronavirus vaccine (CHO cell) group
Experimental: Adult Group 1a
Adult healthy subjects (18 to 59 years of age, inclusive) receive 202-CoV low adjuvant dose at Day 0 and Day 28
Intervention: Biological: 202-CoV low adjuvant dose
Experimental: Adult Group 1b
Experimental: Adult Group 2a
Adult healthy subjects (18 to 59 years of age, inclusive) receive 202-CoV low adjuvant dose at Day 0 and Day 28
Intervention: Biological: 202-CoV low adjuvant dose
Experimental: Adult Group 2b
Biological: Experimental Group1
The COVID-19 vaccine was manufactured by Sinovac Research & Development Co., Ltd,and the quadrivalent influenza vaccine and the 23-valent pneumococcal polysaccharide vaccine(PPV23) were manufactured by Sinovac Biotech Co. The COVID-19 vaccine:600SU inactivated virus in 0.5 mL of aluminium hydroxide solution per
Experimental: candidate vaccine
Intervention: Biological: SARS-CoV-2 Vaccine (Vero Cells), Inactivated
Placebo Comparator: Placebo
Intervention: Biological: Placebo
Biological: SARS-CoV-2 Vaccine (Vero Cells), Inactivated
2 doses of SARS-CoV-2 Vaccine (Vero Cells), Inactivated should be administered as an intramuscular injection into the lateral deltoid of the upper arm with a 28-day interval.
Biological: Placebo
2 doses of Placebo should be administered as an intramuscular injection into the lateral
Experimental: Inactivated SARS-CoV-2 vaccine Lot 1
Participants (n=360) aged 26-45 years will receive Inactivated SARS-CoV-2 vaccine Lot 1 according to 0,28-day immunization schedule.
Intervention: Biological: Inactivated SARS-CoV-2 Vaccine (Vero cell)
Experimental: Inactivated SARS-CoV-2 vaccine Lot 2

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Experimental: Low-dose vaccine (6-11 years)
three doses of low-dose Recombinant COVID-19 vaccine (Sf9 cells) at the schedule of day 0, 21 , 42.
Intervention: Biological: Recombinant COVID-19 vaccine (Sf9 cells)
Experimental: Medium-dose vaccine (6-11 years)

Adults (18-59 years old) A:They were randomly assigned to trial group and placebo group in a ratio of 4:1;Adults (18-59 years old) B:They were randomly assigned to trial group and placebo group in a ratio of 4:1;Adults (18-59 years old) C:They were randomly assigned to trial group and placebo group in a ratio of 4:1;Elderly group (>=60 years old) A:They were randomly assigned to trial group and placebo group in a ratio of 4:1;Elderly group (>=60

1:Internal administration of rehabilitation traditional Chinese medicine, kung fu exercise;2:Internal administration of rehabilitation traditional Chinese medicine;3:No;

control group:Ganmai Dazao Decoction;Experimental group:Dialectical governance based on three parties;

Experimental group:Basic treatment + Fuzheng detoxification granules;Control group:Basic treatment;

Experimental group:Using Fuzheng Yiqing decoction;Parallel control group:Without using Fuzheng Yiqing decoction;External literature data control:None;

18-59 years:Investigational vaccine or placebo at the dose of 5×10^10vp;>=60 years:Investigational vaccine or placebo at the dose of 5×10^10vp;

Phase I (Phase I):Novel coronavirus mRNA vaccine 15ug/0.5ml;Phase II (Phase II) Group A:Novel coronavirus mRNA vaccine 15ug/0.5ml;Phase II (Phase II) Group B:Placebo (normal saline);

Arm A:prompt vaccination strategies;Arm B:deferred vaccination strategies;

Adult low-dose group:Investigational vaccine or placebo of corresponding dose;Elderly low-dose group:Investigational vaccine or placebo of corresponding dose;Adult medium-dose group:Investigational vaccine or placebo of corresponding dose;Elderly medium-dose group:Investigational vaccine or placebo of corresponding dose;Adult high-dose group:Investigational vaccine or placebo of corresponding dose;Elderly high-dose

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test group:mRNA vaccine 15ug/0.5ml;Control group:Placebo 0.5ml;

rehabilitation group:telerehabilitation plus health education ;TCM group:Traditional Chinese medicine plus health education ;control group:health education;

Experimental: Arbidol tablets + basic treatment
Interventions:
Drug: Arbidol
Other: basic treatment
Sham Comparator: basic treatment
Experimental: Colchicine group
The colchicine treatment includes an initial dose of 1 mg (1 mg and 0.5 mg two hours after), followed by 0.5 mg every 12 hours during the next 6 days and 0.5 mg every 24 hours until the completion of 10 days of total treatment.

Control group:conventional treatment;TCMR group:acupressure+Qigong exercise+conventional treatment;

Active Comparator: Inactivated vaccine group
Subjects who have been vaccinated with two doses of inactivated SARS-CoV-2 vaccine will receive one dose of inactivated SARS-CoV-2 vaccine
Intervention: Biological: inactive SARS-CoV-2 vaccine (Vero cell)
Experimental: Low dose aerosolized Ad5-nCoV group

exercise training group:exercise training plus educational instructions;traditional Chinese medicine group:traditional Chinese medicine plus educational instructions;control group:educational instructions;

Experimental: heterologous boost arm with Ad5 vectored vaccine
Subjects who have been primed with two doses of inactive SARS-CoV-2 vaccine will receive a booster of recombinant SARS-CoV-2 Ad5 vectored vaccine after 3~6 months.
Intervention: Biological: Recombinant SARS-CoV-2 Ad5 vectored vaccine
Active Comparator: homogeneous boost arm with inactive vaccine
Experimental: Experimental Group
200 subjects in group A1 will receive one dose of booster immunization 3 months after the completion of the second dose of primary immunization of the SARS-CoV-2 Inactivated vaccine.
Intervention: Biological: SARS-CoV-2 Inactivated Vaccine
Experimental: Experimental group
Experimental Group (468 subjects) will receive: 1st dose : combined vaccination of COVAX+PPV23; 2nd dose: combined vaccination of COVAX+IIV4
Intervention: Biological: COVAX+PPV23;COVAX+IIV4
Active Comparator: Control group A

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Experimental: JS016 treatment group
Standard therapy + JS016 injection Standard therapy including vital sign monitoring, supplementary oxygen and respiratory support in accordance with the patient's oxygen saturation and respiratory condition.
Intervention: Drug: JS016
Experimental: Experimental Group
600 participants will receive one dose of booster vaccination with high-dosage inactivated SARS-CoV-2 vaccine .
Intervention: Biological: High-dosage SARS-CoV-2 vaccine
Active Comparator: Control Group
Experimental: Population I
3 doses of experimental vaccine (pilot scale batch) were given at day 0, 30 and 60.
Intervention: Biological: Recombinant new coronavirus vaccine (CHO cell) group
Experimental: Population II
3 doses of experimental vaccine (process validation lot 1) at day 0, 30 and 60;

control group:Follow-up and lifestyle guidance;experimental group:rehabilitation training;

Oral leflunomide + inhaled interferon alpha group:Oral leflunomide + inhaled interferon alpha ;inhaled interferon alpha group:inhaled interferon alpha;

Adults (18-59 years old):Subjects were randomly assigned to the low-dose, high-dose, and placebo groups in a ratio of 2:2:1 (the low-dose and high-dose groups were the medium and high-dose groups in the Phase I clinical trial). All subjects received one dose of the trial vaccine or placebo (an excipient without antigen).;Elderly (>=60 years old):Subjects were randomly assigned to the low-dose, high-dose, and placebo groups in a ratio of 2:2:1 (the

A group:Recombinant COVID-19 vaccine (CHO cell);B group:COVID-19 inactivated vaccine (Vero cell);

Experimental Group:FNC+Symptomatic treatment (except antiviral treatment);Control group:Antiviral therapy + symptomatic treatment;

conventional treatment group:conventional treatment;conventional treatment combined with hymecromone tablets group:conventional treatment combined with hymecromone tablets treatment;

Experimental: Population I
One dose of influenza virus split vaccine and one dose of recombinant Novel coronavirus vaccine (CHO cells) were given in the contralateral arm on day 0, the second dose of recombinant Novel coronavirus vaccine (CHO cells) on day 30 and the third dose of recombinant Novel coronavirus vaccine (CHO cells) on day 60, all in the upper arm deltoid

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test group (18-59 years):SARS-CoV-2mRNA vaccine 15ug/0.5ml;control group (18-59 years):COVID-19 Vaccine (Vero Cell), Inactivated;test group (>=60years):SARS-CoV-2mRNA vaccine 15ug/0.5ml;control group (>=60years):COVID-19 Vaccine (Vero Cell), Inactivated;

Experimental: Experimental Group

The experimental group is also called the combined immunization group.260 participants will receive the first dose of COVID-19 vaccine and EV71 vaccine on day 0 and the second dose of COVID-19 vaccine and EV71 vaccine on day 28.

Intervention: Biological: Experimental Group

A1: pGX9501 (0.5 mg)/placebo;A2:pGX9501 (1.0 mg)/placebo;A3:pGX9501 (2.0 mg)/placebo;B1:pGX9501 (0.5 mg)/placebo;B2:pGX9501 (1.0 mg)/placebo;B3:pGX9501 (2.0 mg)/placebo;

Lianhua Qingwen Group:Capsules 4 capsules/time, 3 times a day;Basic treatment group:In principle, no drugs and other treatments are used;

Experimental: Inactivated SARS-CoV-2 vaccine Lot 1 of the workshop 2

360 Participants aged 3-17 years(including 52 children aged 3-5 years,154 children aged 6-11 years and 154 adolescents aged 12-17 years) will receive Inactivated SARS-CoV-2 vaccine Lot 1 of the workshop 2 according to 0,28-day immunization schedule.

Intervention: Biological: Inactivated SARS-CoV-2 Vaccine (Vero cell) Lot 1 of the workshop

Experimental: Investigational vaccine group 1

100 tuberculosis patients will receive two doses of standard dosage CoronaVac plus one dose of double dosage CoronaVac at a schedule of 0, 28, 56 days.

Interventions:

Biological: Standard dosage inactivated vaccine

Experimental: Experimental Group 1

96 subjects who completed primary immunization with inactivated COVID-19 vaccine manufactured by Sinovac Research & Development Co.,Ltd. will receive one dose of booster immunization with inactivated COVID-19 vaccine manufactured by Sinovac Research & Development Co.,Ltd.

Experimental: Experimental Group-High-dosage of COVID-19 vaccine (Vero cell), Inactivated

170 participants will receive one dose of booster vaccination with high-dosage of COVID-19 vaccine (Vero cell), Inactivated.

Intervention: Biological: High-dosage of COVID-19 vaccine (Vero cell), Inactivated

Experimental: IBI314

Low/medium/high dose, intravenously, once, on Day 1

Interventions:

Biological: IBI314(low dose)

Biological: IBI314(high dose)

18 to 59 years Group:Administration of the investigational vaccine (5×10^10vp) or placebo;>=60 years Group:Administration of the investigational vaccine (5×10^10vp) or placebo;

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Placebo Comparator: JS026/placebo 30 mg
4 patients will be enrolled in this arm.
Interventions:
Biological: JS026/placebo
Biological: JS016/placebo
Experimental: Probiotics
The intervention group consumes oral probiotics 1 time/day for 7 consecutive days.
Intervention: Dietary Supplement: Probiotics daily
Placebo Comparator: Placebos
The placebo group consumes oral placebo 1 time/day for 7 consecutive days
Experimental: Probiotics-Placebo
1 week of consecutively consuming oral probiotics (1 time/day) followed by SARS-CoV-2 vaccination, then 2 weeks of consecutively consuming oral placebo (1 time/day).
Interventions:
Dietary Supplement: 1 week of probiotics(Week 1)

1: Jinshuibao tablet;
2: Placebo;

Vaccine: Three doses of experimental vaccine at the schedule of day 0, 28, 56;
Placebo: Three doses of placebo at the schedule of day 0, 28, 56;
Vaccine(China cohort, for immunogenicity bridging): Three doses of experimental vaccine at the schedule of day 0, 28, 56
1 (inactivated vaccine group): subjects were injected with 1 dose of inactivated COVID-19 vaccine (intramuscular injection) at the baseline (day 0);
2 (recombinant protein vaccine group): the subjects were injected with 1 dose of recombinant protein vaccine (intramuscular injection) at the baseline (day 0):
Control group: no intervention

Test group: SARS-CoV-2 mRNA vaccine 15ug/0.5ml
Control group: Novel Coronavirus Inactivated Vaccine (Vero Cells)

Intervention group: MBSR
Control group: Routine care

A1:pGX9501 1.0 mg; A2:pGX9501 2.0 mg; A3:placebo;A4:placebo

Experimental: Group A
Subjects in safety cohort will receive one heterologous booster dose of aerosolized Ad5-nCoV
Intervention: Biological: Aerosolized Ad5-nCoV
Experimental: Group B

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Single-dose A
A single dose A of FB2001 or placebo will be administered by intravenous (IV) infusion
Interventions:
Drug: FB2001
Other: FB2001 placebo

No Intervention: Control Group; Experimental: Inactivated Vaccine Group; Experimental: Recombinant subunit protein vaccine: Experimental: mRNA vaccine Group

Experimental: 18~59 yrs. low dosage (20 μg) - SCTV01C VACCINE
20 participants in Phase I and 120 participants in Phase II at the age of 18~59 years old will receive two doses of low dosage (20 μg/0.5mL) SCTV01C VACCINE on Day 0 and Day 28
Intervention: Biological: SCTV01C

Experimental: AZD7442: co-administration of a single dose of 600 mg AZD7442 (300 mg AZD8895 and 300 mg AZD1061) by intravenous (IV) infusion.;
Placebo Comparator: Placebo: co-administration of a single dose of 600mg placebo by intravenous (IV) infusion.;

control group: routine prone positioning
intervention group: prone positioning+nursing exercise intervention

(Experimental group) Respiratory function exercise combined with normal drug treatment group: Respiratory exercise combined with medication
(Control group) Drug treatment group: medical treatement

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Funding	Primary Sponsor	Registration Status (According to First Enrollment Date and Date of First Submission)
Min	Wuhan Jinyintan Hospital (Wuhan Infectious Diseases Hospital)	Retrospective
#NOTREQ	Peking Union Medical College Hospital	Prospective
#NOTREQ	Beijing 302 Hospital	Retrospective
Chongqing Science and Technology of Bureau	Chongqing Public Health Medical Center	Prospective
Chongqing Science and Technology Bureau	Chongqing Public Health Medical Center	Retrospective
#NOTREQ	Shanghai Public Health Clinical Center	Prospective
Beijing University of Chinese Medicine, Dongzhimen	Dongzhimen Hospital Affiliated to Beijing University of Chinese Medicine	Prospective
#NOTREQ	The Second Hospital of Nanjing Medical University	Prospective
#NOTREQ	Guangzhou 8th People's Hospital	Retrospective

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#NOTREQ	Capital Medical University	Prospective
#NOTREQ	Capital Medical University	Prospective
Self-raised	Affiliated Zhongshan Hospital of Dalian University	Retrospective
Shijiazhuang Yiling Pharmaceutical Co.Ltd.	Hebei Yiling Hospital、Renmin Hospital of Wuhan University	Prospective
Government-funded research projects ; Shijiazhuang	Hebei Yiling Hospital, Renmin Hospital of Wuhan University	Prospective
Wuhan 1st Hospital	Wuhan 1st Hospital	Prospective
MOST	Beijing hospital of Traditional Chinese medicine; Hubei integrated traditional Chinese and Western Medicine Hospital	Prospective
MOST	Beijing Hospital of Traditional Chinese Medicine affiliated to Capital Medical University/Beijing institute of Traditional Chinese	Prospective
self-financing	1.Xinhua affiliated hospital, Hubei University of Chinese Medicine; 2. Hubei Provincial Hospital of Integrated Chinese and Western	Prospective
self-financing	1. Xinhua affiliated hospital, Hubei University of Chinese Medicine; 2. Hubei Provincial Hospital of Integrated Chinese and Western	Prospective

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Self-financing	1. Xinnua affiliated hospital, Hubei University of Chinese Medicine; 2. Hubei Provincial Hospital of Integrated Chinese and Western	Prospective
self-financing	1. Xinnua affiliated hospital, Hubei University of Chinese Medicine; 2. Hubei Provincial Hospital of Integrated Chinese and Western	Prospective
Self-financing	1. Xinnua Affiliated Hospital, Hubei University of Chinese Medicine; 2. Hubei Provincial Hospital of Integrated Chinese and Western	Prospective
#NOTREQ	Tongji Hospital	Retrospective
#NOTREQ	Tongji Hospital	Retrospective
#NOTREQ	Tongji Hospital	Retrospective
Self financing and application for scientific research funds	The First Hospital of Changsha; The Second Xiangya Hospital of Central South University	Retrospective
Self-financing	Zhejiang Chinese Medical University	Prospective
Self-financing	Zhejiang Chinese Medical University	Prospective
Self-financing	Tongji Hospital, Tongji Medical College, Huazhong University of Science and Technology	Prospective

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self-financing	Zhongnan Hospital of Wuhan University	Prospective
Self-financing	The First Hospital Affiliated to Zhejiang University's Medical School	Prospective
Self-financing	The First Affiliated Hospital, Zhejiang University School of Medicine	Prospective
Science & Technology Department of Sichuan Province	Hospital of Chengdu University of Traditional Chinese Medicine	Retrospective
Government	Renmin Hospital of Wuhan University	Retrospective
funding	Xiangyang 1st People's Hospital	Prospective
#NOTREQ	Tongji Hospital	Retrospective
#NOTREQ	ZhiYong Peng	Prospective
funding	Xiangyang First People's Hospital	Prospective
coordinater	The First Affiliated Hospital of Medical College of Zhejiang University	Prospective

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Tongji Hospital, Tongji Medical College, Huazhong University of	Department of Hematology, Tongji Hospital, Tongji Medical College, Huazhong University of Science and Technology	Retrospective
raise independently	Beijing hospital of Traditional Chinese Medicine	Prospective
National Science and Technology Major Project of China	Hubei Provincial Hospital of TCM	Retrospective
Project of State Administration of Traditional Chinese Medicine of	Hubei Provincial Hospital of TCM	Retrospective
Self-financing	The First Affiliated Hospital of Zhejiang University School of Medicine	Prospective
Harbin Pharmaceutical Group Sanjing Pharmaceutical Holding Co. Ltd	Tongji Hospital, Tongji Medical College, Huazhong University of Science and Technology	Prospective
#NOTREQ	Shanghai Public Health Clinical Center	Prospective
Emergency Project of Science Technology Department of	The First Affiliated Hospital, College of Medicine, Zhejiang University	Retrospective
Science and Technology Project of Novel Coronavirus Pneumonia in	West China Hospital, Sichuan University	Prospective
Emergency Project for 2019-nCoV of Ministry of Science and	Wuhan Pulmonary Hosptial	Prospective

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Department of Anesthesiology	Central Theater General Hospital of PLA	Prospective
Government	The First Hospital of Peking University	Prospective
Sun Yat-Sen University	The Fifth Affiliated Hospital Sun Yat-Sen University	Prospective
Tongji Hospital,Tongji Medical College, Huazhong	Tongji Hospital,Tongji Medical College, Huazhong University of Science and Technology	Retrospective
Department of Science and Technology of Anhui Province	The First Affiliated Hospital of Anhui University of Traditional Chinese Medicine	Prospective
Tongji Hospital	Tongji Hospital, Tongji Medical College of HUST	Prospective
spontaneous	Renmin Hospital of Wuhan University	Prospective
Government	China-Japan friendship hospital	Prospective
self-financing	The Second Affiliated Hospital of Chongqing Medical University	Prospective
Chonqing Medical University	The Second Affiliated Hospital of Chongqing Medical University	Prospective

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Chongqing Municipal Health Committee	The First Affiliated Hospital of Chongqing Medical University	Prospective
Chongqing Municipal Health Committee	The First Affiliated Hospital of Chongqing Medical University	Prospective
National Key R&D Program of China (No.2020YFC0841500);	China Academy of Chinese Medical Sciences; Zhongnan Hospital of Wuhan University	Prospective
Department of science and technology of Anhui Province	The First Affiliated Hospital of University of science and technology of China (Anhui Provincial Hospital)	Retrospective
Self-financing	Zhongnan Hospital of Wuhan University	Retrospective
Taizhou Science and Technology Plan Project	Enze Hospital of Taizhou Enze Medical Center (Group)	Prospective
self-raised	The First Affiliated of Wenzhou Medical University	Retrospective
Emergency Committee of the World Federation of Chinese	Huangshi Hospital of Traditional Chinese Medicine	Retrospective
Lizhu Group Limin Pharmaceutical Factory	Union Hospital affiliated to Tongji Medical College of Huazhong University of Science and Technology	Prospective
Sichuan Guangda Pharmacy Co., Ltd.	Union Hospital affiliated to Tongji Medical College of Huazhong University of Science and Technology	Prospective

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Horizontal project support	Dongzhimen Hospital Affiliated to Beijing University of Chinese Medicine	Prospective
Central Plains Scholar Scientist Studio Project	The First Affiliated Hospital of He'nan University of Chinese Medicine	Prospective
Tongji University	Shanghai Pulmonary Hospital	Prospective
#NOTREQ	Xinxiang medical university	Prospective
self-raised	Renmin Hospital of Wuhan University	Prospective
Ministry of Science and Technology of China	Wuhan Jinyintan Hospital (Wuhan Infectious Diseases Hospital)	Retrospective
Shangnai Municipal Development & Reform Commission	Shanghai Public Health Clinical Center	Prospective
self	Nanjing Second Hospital	Retrospective
Guangznou Regenerative Medicine and Health Guangdong	Jingzhou Central Hospital	Prospective
#NOTREQ	Wuhan Union Hospital, China	Prospective

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#NOTREQ	Second Affiliated Hospital of Wenzhou Medical University	Prospective
Guangznou Regenerative Medicine and Health Guangdong	Jingzhou Central Hospital	Prospective
Science and Technology Department of He'nan Province	The First Affiliated Hospital of Zhengzhou University	Retrospective
Fudan University	Zhongshan Hospital, Fudan University	Prospective
self-raised	People's Hospital of Guangshan County	Prospective
raise independently	The First Affiliated Hospital of Medical College of Zhejiang University	Prospective
Ministry of Science and Technology of the People's Republic of	Beijing You'an Hospital, Capital Medical University	Prospective
SPH SHANGHAI ZHONGXI PHARMACEUTICAL CO., LTD	Ruijin Hospital, Shanghai Jiaotong University School of Medicine	Retrospective
Emergency Committee of the World Federation of Chinese	Huangshi Hospital of Traditional Chinese Medicine	Retrospective
#NOTREQ	Beijing Chao Yang Hospital	Retrospective

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#NOTREQ	ShuGuang Hospital	Prospective
Peking University Third Hospital self-funds	Peking University Third Hospital	Prospective
Peking University Third Hospital self-fund	Peking University Third Hospital	Prospective
HwaMei Key Research Fund (2020HMZD18)	HwaMei Hospital, University of Chinese Academy of Sciences	Retrospective
Ministry of Science and Technology, China；Fund of Shanghai	Longhua Hospital Affiliated to Shanghai University of Traditional Chinese Medicine	Prospective
Ministry of Science and Technology, China; Fund of Shanghai	Longhua Hospital Affiliated to Shanghai University of Traditional Chinese Medicine	Prospective
spontaneous	Hubei Hospital of Traditional Chinese Medicine	Prospective
Science & Technology Department of Sichuan Province	Hospital of Chengdu University of Traditional Chinese Medicine	Prospective
Self-financing	Tongji Hospital, Tongji Medical College,Huazhong University of Science and Technology	Prospective
Health Commission of Shandong Province, Department of	Qingdao East Sea Pharmaceutical Co., Ltd.	Retrospective

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Emergency Research Project of Shanghai University of	Yueyang Hospital of Integrated Traditional Chinese and Western Medicine, Shanghai University of Traditional	Prospective
Hubei Clinical Research Center for Emergency and Resuscitation	Zhongnan Hospital of Wuhan University	Retrospective
Hospital emergency research project	Chinese PLA General Hospital	Prospective
The National Key Research and Development Program of	Institute of Basic Medical Sciences, Chinese Academy of Medical Sciences	Retrospective
Self-financing	The First Affiliated Hospital of Nanchang University	Prospective
Xiamen science and Technology Bureau	Zhongshan Hospital Affiliated to Xiamen University	Retrospective
Shijiaznuang Pharmaceutical Group Co., Ltd./ Lei Yunshang Pharmaceutical	The First Affiliated Hospital of Guangzhou Medical University	Prospective
Shanghai University of TCM research program for 2019-ncov	Shanghai University of TCM	Prospective
Health commission of Heilongjiang province	Health commission of Heilongjiang province	Retrospective
Anhui Development and Reform Commission	The First Affiliated Hospital of University of science and technology of China (Anhui Provincial Hospital)	Retrospective

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Science & Technology Department of Sichuan Province	Hospital of Chengdu University of Traditional Chinese Medicine	Prospective
National Key Research and Development Project (2018YFC2001	Union Hospital, Tongji Medical College, Huazhong University of Science and Technology	Prospective
Self financing	The First Affiliated Hospital of Guangzhou Medical University	Retrospective
Sinopharm Wuhan Blood Products Co., Ltd.	Wuhan Jinyintan Hospital (Wuhan Infectious Diseases Hospital)	Prospective
Evaluation of efficacy of mycobacterium vacce for injection for	Guangxi medical uniwresity	Retrospective
#NOTREQ	China Academy of Chinese Medical Sciences	Prospective
Self-raised	Beijing Chaoyang Hospital, Capital Medical University	Prospective
Self-funding	Wuhan Fourth Hospital	Retrospective
Self-financing	First Affiliated Hospital of Anhui Medical University	Prospective
Self-financing	West China Hospital	Prospective

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Guangznou Regenerative Medicine and Health Guangdong	The Sixth Affiliated Hospital of Guangzhou Medical University (Qingyuan People's Hospital)	Retrospective
The project of the First Affiliated Hospital of Guangzhou	The First Affiliated Hospital of Guangzhou Medical University	Prospective
Science Technology Department of Zhejiang Province	Tongde Hospital of Zhejiang Province	Retrospective
Researcher self-financing	Peking University Third Hospital	Prospective
#NOTREQ	The First Affiliated Hospital of Guangzhou Medical University	Retrospective
Xiamen science and Technology Bureau	Zhongshan Hospital Affiliated to Xiamen University	Prospective
Major regional immunity cultivation project, 91742109	The First Affiliated Hospital of Guangzhou Medical University	Retrospective
Science and technology department of Hubei province	Renmin Hospital of Wuhan University	Prospective
Government	The First Affiliated Hospital of Nanchang University	Prospective
The First Hospital of Shanxi Medical University (136 medical	The First Hospital of Shanxi Medical University	Prospective

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Funding for emergency research projects	The Sixth Medical Center of PLA General Hospital	Prospective
Shangnai Changzheng Hospital Project Fund and Drug donation from	Shanghai Changzheng Hospital	Prospective
self-financing	Tianjin Huanhu Hospital	Prospective
Financial Appropriation of Jiangxi Provincial Department of	The First Affiliated Hospital of Nanchang University	Retrospective
Self-financing	Shanghai Public Health Clinical Center	Retrospective
Shenzhen Science and Technology Bureau	Shenzhen national medicine inheritance medical research institute co. LTD	Prospective
self-financing	The Second Affiliated Hospital Of Xi'an Jiaotong University	Prospective
Chinese PLA General Hospital	Chinese PLA General Hospital	Prospective
COVID-19 emergency research project of Chinese PLA general hospital	The 5th Medical Center Chinese PLA General Hospital	Prospective
Self-funded	Renmin Hospital of Wuhan University	Prospective

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Government support and enterprise self-financing	Hunan yuanpin Cell Biotechnology Co., Ltd	Retrospective
Raise independently	The First Affiliated Hospital of Nanchang University	Prospective
Self financing	Jingzhou First People's Hospital	Prospective
自筹	Guangdong Provincial Hospital of Chinese Medicine - Zhuhai Hospital	Retrospective
#NOTREQ	Beijing 302 Hospital	Prospective
#NOTREQ	Puren Hospital Affiliated to Wuhan University of Science and Technology	Prospective
Scientific research project on new coronavirus pneumonia in	West China Hospital of Sichuan University	Retrospective
Not stated	The First Affiliated Hospital of Nanchang University, Ten layers pulse Research Institute of traditional Chinese medicine	Prospective
Ganzhou Science and Technology Bureau	Fifth People's Hospital of Ganzhou	Retrospective
other	Union Hospital, Tongji Medical College, Huazhong University of Science and Technology	Prospective

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Self financing	Yueyang Hospital of Integrated Traditional Chinese and Western Medicine, Shanghai University of Traditional	Retrospective
China National Center for Biotechnology Development	Zhongnan Hospital of Wuhan University	Retrospective
SPH Xing Ling Sci. & Tech. Pharmaceutical Co.,Ltd	Shanghai University of Traditional Chinese Medicine	Prospective
Ascletis Pharmaceuticals CO., LTD.	Shanghai Changzheng Hospital	Retrospective
Raise independently	Ganzi Hospital of West China Hospital, Sichuan University	Prospective
Wuxi novel coronavirus infection emergency control	Wuxi Fifth People's Hospital	Prospective
Novel coronavirus pneumonia clinical optimization	The First Affiliated Hospital of Harbin Medical University	Retrospective
National Key R&D Program of China (No.2020YFC0841500);	China Academy of Chinese Medical Sciences	Prospective
Tongji Hospital of Tongji Medical College; Huazhong	Tongji Hospital of Tongji Medical College; Huazhong Science and Technology University	Prospective
Project leader's SCI funds	West China Hospital of Sichuan University	Prospective

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NO	Union Hospital, Tongji Medical College, Huazhong University of Science & Technology	Prospective
Xi'an Medical university	The Second Affiliated Hospital of Xi'an Medical University	Prospective
#NOTREQ	Ansun Biopharma, Inc.	Prospective
Sinopharm Wuhan Blood Products Co., Ltd.	First people's hospital of Jiangxi district，Wuhan	Prospective
raise independently	Jingzhou First People's Hospital	Retrospective
Government, foundation, company	Wuhan Jinyintan Hospital (Wuhan Infectious Diseases Hospital)	Retrospective
raise independently	Harbin infectious diseases hospital	Prospective
self-finance	The First Affiliated Hospital of Fujian Medical University	Prospective
Liaoning province science and technology plan project	Institute of Integrative Medicine of Dalian Medical University, Dalian, China.	Prospective
Optional subjects	The First Affiliated Hospital of Fujian Medical University	Prospective

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Department of Anesthesiology	The First Affiliated Hospital of Guangzhou University of TCM	Prospective
Liaoning province science and technology plan project	Dalian Medical University, Dalian, China.	Retrospective
Shanghai University of T.C.M.	Shanghai University of TCM	Retrospective
Self-funded	Maoming People's Hospital	Prospective
Prevention and treatment of novel coronavirus by Shanghai	Cancer Institute, Longhua Hospital, Shanghai University of Traditional Chinese Medicine	Prospective
Self-financing	Affiliated Hospital of traditional Chinese and Western Medicine Nanjing University of Chinese Medicine	Retrospective
Self-financing	Affiliated Hospital of traditional Chinese and Western Medicine Nanjing University of Chinese Medicine	Retrospective
Tongji Hospital of Tongji Medical College, Huazhong	Department of Respiratory and Critical Care Medicine, Tongji Hospital of Tongji Medical College, Huazhong University of Science and	Prospective
Science and Technology Department of Hubei Province	Zhongnan Hospital of Wuhan University	Retrospective
-	The first affiliated hospital of guangzhou university of Chinese medicine	Retrospective

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Beijing Darwin Cell Biotechnology Co., Ltd	Hubei Shiyan Taihe hospital	Retrospective
#NOTREQ	Henan Provincial People's Hospital	Prospective
self-financing	The First Hospital of Harbin Medical University	Prospective
Self-financing	The Second Affiliated Hospital of Wenzhou Medical University	Retrospective
Project of Sichuan Provincial Department of Science and	Affiliated Hospital of Chengdu University of Traditional Chinese Medicine	Prospective
self-funding	Wuhan Red Cross Hospital	Retrospective
Huaznong University of Science and Technology COVID-19	Tongji Hospital Affiliated to Tongji Medical College of Huazhong University of science and technology	Prospective
spontaneous	Hubei Hospital of Traditional Chinese Medicine	Retrospective
self-raising	Shandong Provincial Hospital	Retrospective
supported by Zhejiang University special scientific	Shanghai General Hospital	Retrospective

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Science and Technology Department of He'nan Province	The First Affiliated Hospital of Zhengzhou University	Retrospective
#NOTREQ	Peking University First Hospital	Retrospective
The investigator will personally pay the additional costs of normal	Liaocheng People's Hospital	Prospective
#NOTREQ	Qilu Hospital of Shandong University	Prospective
Self-raised	Union Hospital affiliated to Tongji Medical College of Huazhong University of Science and Technology	Retrospective
Self-raised	The Third People's Hospital of Shenzhen	Prospective
Government	China-Japan friendship hospital	Retrospective
National project	Xiangya Hospital of Central South University	Prospective
Raise independently	The First People's Hospital of Jiangxia District, Wuhan	Retrospective
Hubei Clinical Research Center for Emergency and Resuscitation	Zhongnan Hospital of Wuhan University	Retrospective

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National Science and Technology Major Project of China	Hubei Provincial Hospital of TCM	Retrospective
Apply for special fund support	Shenzhen Third People's Hospital	Retrospective
Wenzhou science and Technology Bureau	The First Affiliated Hospital of Wenzhou Medical University	Retrospective
Shangnai Changzheng Hospital Project Fund and Drug donation from	Shanghai Changzheng Hospital	Prospective
Emergent Technology Project of Maoming	Maoming People's Hospital	Retrospective
#NOTREQ	Zhong Wang	Prospective
Beijing continent Pharmaceutical Company Limited, 120000	State Key Laboratory of Respiratory Disease, National Clinical Center for Respiratory Disease, Guangzhou Institute of	Retrospective
2020YJY0202	HwaMei Hospital, University of Chinese Academy of Sciences	Retrospective
2020YJY0201	HwaMei Hospital, University of Chinese Academy of Sciences	Retrospective
Self-financing	Foshan First People's Hospital	Retrospective

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Department of science and technology of Shaanxi Province	Affiliated Hospital of Shaanxi University of Traditional Chinese Medicine	Retrospective
Sinopharm Wuhan Blood Products Co., Ltd.	Renmin Hospital of Wuhan University	Prospective
Joint project of Science and Technology Department of Jiangxi	The First Affiliated Hospital of Nanchang University	Prospective
Guangznou Baiyunshan Jingxiutang Pharmaceutical Co., Ltd	The First Affiliated Hospital of Guangzhou Medical University	Prospective
Funded by the Science and Technology departments of Jiangxi	The Second Affiliated Hospital of Nanchang University	Prospective
#NOTREQ	Beijing Chao Yang Hospital	Retrospective
National key R & D plan 2020YFC0841500	Guangdong Provincial Hospital of Chinese Medicine	Prospective
Basic scientific research business expenses of central level	Xiyuan Hospital, Chinese Academy of Traditional Chinese Medicine	Retrospective
Funding of Brand Subjects of Ningbo Kangning Hospital	Ningbo Kangning Hospital	Prospective
Science and Technology Project Fund of Changsha Science and	The third xiangya hospital of Central South University	Retrospective

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Shanghai Key Discipline Construction-Infectious Diseases	Shanghai Public Health Clinical Center	Prospective
#NOTREQ	Guangzhou Institute of Respiratory Disease	Prospective
The National Science and Technology Major Projects for	Beijing you'an Hospital, Capital Medical University	Retrospective
#NOTREQ	Shanghai Asclepius Meditec Inc.	Retrospective
#NOTREQ	Chongqing Public Health Medical Center	Retrospective
#NOTREQ	Fuzhou General Hospital	Retrospective
Three major scientific research projects of Sun Yat-sen	The Third Affiliated Hospital of Sun Yat-sen University	Prospective
The Pioneering Program of the First Teaching Hospital of Tianjin	First Teaching Hospital of Tianjin University of Traditional Chinese Medicine	Retrospective
#NOTREQ	Renmin Hospital of Wuhan University	Prospective
Tongji Hospital of Tongji Medical College, Huazhong	Tongji Hospital of Tongji Medical College, Huazhong Science and Technology University	Prospective

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government	Infection prevention and treatment center of The Fifth Affiliated Hospital of Sun Yat-Sen University	Retrospective
#NOTREQ	Peking University First Hospital	Prospective
#NOTREQ	Peking University First Hospital	Prospective
Shanghai sixth people's hospital	Shanghai Sixth People's Hospital	Retrospective
Novel coronavirus pneumonia emergency project of	Beijing University of Chinese Medicine	Retrospective
#NOTREQ	Institute of Biotechnology, Academy of Military Medical Sciences, PLA of China	Prospective
Huzhou science and technology bureau	Huzhou Central Hospital	Retrospective
Ministry of Science and Technology, China	Henan Provincial Center for Disease Control and Prevention	Prospective
Jiangsu CF PharmTech, Inc.	Jiangsu Province Hospital	Prospective
self-finance	Guangzhou Eighth People's Hospital	Retrospective

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#NOTREQ	Sinovac Research and Development Co., Ltd.	Prospective
National Key R&D Program of China (No.2020YFC0841500)	China Academy of Chinese Medical Sciences	Prospective
emergency special project of the Science and Technology Ministry in	Beijing University of traditional Chinese Medicine	Retrospective
Logistics research Project	Sixth Medical Center of PLA General Hospital	Prospective
Shanghai Public Health Clinical Center	Shanghai Public Health Clinical Center	Prospective
#NOTREQ	Guang'anmen Hospital of China Academy of Chinese Medical Sciences	Prospective
National Key Research and Development Project	Hubei Provincial Hospital of TCM	Prospective
Science and Technology Department of State Administration	Wuhan Hospital Of Traditional Chinese and Western Medicine (Wuhan 1st Hospital)	Retrospective
Ministry of science and technology	Hubei Provincial Hospital of TCM	Prospective
Novel coronavirus pneumonia clinical optimization	Mudanjiang Kang'an Hospital	Prospective

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#NOTREQ	Xijing Hospital	Retrospective
Self-financing	Beijing University of Chinese Medicine Third Affiliated Hospital	Prospective
West China Hospital, Sichuan University	West China Hospital, Sichuan University	Retrospective
Anti-epidemic emergency research project of Shanghai University of	Yueyang Integrated Traditional Chinese and Western Medicine Hospital Affiliated to Shanghai University of Traditional	Retrospective
Ministry of science and technology	Xiaogan Traditional Chinese Medicine Hospital	Retrospective
None	West Campus of Union Hospital affiliated to Tongji Medical College of Huazhong University of Science and Technology	Prospective
Ministry of Science and Technology, China	Henan Provincial Center for Disease Control and Prevention	Prospective
Research funds	Shanghai Public Health Clinical Center	Retrospective
National major research and development plan, Key project of	State Key Laboratory of Respiratory Disease, Guangzhou Institute of Respiratory Diseases, the First Affiliated Hospital of	Prospective
National Science and Technology Major Project of China	Hubei Provincial Hospital of TCM	Prospective

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Special Research Project on Prevention and Control of	The First Affiliated Hospital of Guangdong Pharmaceutical University; Guangzhou Eighth People's Hospital	Prospective
The sponsors raise the money themselves	Beijing Ditan Hospital affiliated to Capital Medical University	Prospective
#NOTREQ	Sinovac Research and Development Co., Ltd.	Prospective
#NOTREQ	Chinese Medical Association	Retrospective
Institute of Science and Technology, Guangzhou University of	Hongqi Hospital Affiliated to Mudangjiang Medical University	Retrospective
cience and Technology Department of State Administration	Wuhan Third Hospital	Retrospective
China Medical University	The First Hospital of China Medical University	Retrospective
Harbin Pharmaceutical Group Sanjing Pharmaceutical Holding Co. Ltd	The First Affiliated Hospital of Wenzhou Medical University	Retrospective
#NOTREQ	Chinese Academy of Medical Sciences	Retrospective
No funding.	Jiangsu People's Hospital	Retrospective

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Self-financing	Guangzhou Women and Children's Medical Center	Prospective
self-raised	Xiangyang Central Hospital	Retrospective
Anti-epidemic emergency research project of Shanghai University of	Yueyang Integrated Traditional Chinese and Western Medicine Hospital Affiliated to Shanghai University of Traditional	Retrospective
Shanghai Science and Technology Commission	Department of Pulmonology, Shuguang Hospital Affiliated to Shanghai University of Traditional Chinese Medicine	Retrospective
#NOTREQ	Shanghai Junshi Bioscience Co., Ltd.	Retrospective
Wanbangde Pharmaceutical Group Co.,Ltd.	Children's Hospital Affiliated to Zhejiang University School of Medicine	Retrospective
#NOTREQ	Anhui Zhifei Longcom Biologic Pharmacy Co., Ltd.	Prospective
Bengbu Medical College	Bengbu Medical College	Retrospective
Self-funded	Shulan (Hangzhou) Hospital; Center for Disease Control and Prevention of Guangxi Zhuang Autonomous Region	Prospective
#NOTREQ	Anhui Zhifei Longcom Biologic Pharmacy Co., Ltd.	Prospective

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#NOTREQ	Chinese Academy of Medical Sciences	Retrospective
#NOTREQ	Brii Biosciences Limited	Retrospective
#NOTREQ	Brii Biosciences Limited	Retrospective
#NOTREQ	Sinocelltech Ltd.	Prospective
Wuhan Municipal Health Commission	Hubei Hospital of Traditional Chinese Medicine	Retrospective
Shanghai Public Health Clinical Center	Shanghai Public Health Clinical Center	Prospective
#NOTREQ	Anhui Zhifei Longcom Biologic Pharmacy Co., Ltd.	Prospective
#NOTREQ	BioNTech SE	Retrospective
#NOTREQ	Mabwell (Shanghai) Bioscience Co., Ltd.	Retrospective
#NOTREQ	Jiangsu Province Centers for Disease Control and Prevention	Prospective

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Innovation Fund for Medical Sciences sponsored by Chinese	Chinese Academy of Medical Sciences	Prospective
Self financing	Jiangsu Provincial Center For Disease Control and Prevention (Public Health Research Institute of Jiangsu Province)	Prospective
Sponosor Investment	Huashan Hospital Affiliated to Fudan University	Prospective
#NOTREQ	Sinovac Research and Development Co., Ltd.	Prospective
Cross-topic of special emergency for pneumonia prevention and	The Third Affiliated Hospital of Beijing University of traditional Chinese Medicine	Prospective
Beijing University of Chinese Medicine 2020 Basic Scientific	The Third Affiliated Hospital of Beijing University of traditional Chinese Medicine	Prospective
#NOTREQ	CanSino Biologics Inc.	Prospective
#NOTREQ	Shanghai Junshi Bioscience Co., Ltd.	Prospective
Beijing University of Traditional Chinese Medicine 2020	Beijing University of Chinese Medicine Third Affiliated Hospital	Retrospective
Self-funded	Jiangsu Provincial Center for Disease Control and Prevention(Public Health Research Institute of Jiangsu Province)	Prospective

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#NOTREQ	Guangzhou Institute of Respiratory Disease	Prospective
Self-funded	Shulan (Hangzhou) Hospital	Prospective
Xiangzhou District People's Hospital	Xiangyang Central Hospital	Retrospective
Self-funded	Jiangsu Provincial Center for Disease Control and Prevention (Public Health Research Institute of Jiangsu Province)	Retrospective
#NOTREQ	Sinovac Research and Development Co., Ltd.	Retrospective
Self financing	Jiangsu Provincial Center For Disease Control and Prevention (Public Health Research Institute of Jiangsu Province)	Prospective
#NOTREQ	Mabwell (Shanghai) Bioscience Co., Ltd.	Prospective
#NOTREQ	BioNTech SE	Prospective
#NOTREQ	Jiangsu Province Centers for Disease Control and Prevention	Retrospective
#NOTREQ	Jiangsu Province Centers for Disease Control and Prevention	Retrospective

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Sponosor	Jiangsu Provincial Center for Disease Control and Prevention	Prospective
Self-funded	Hunan Provincial Center for Disease Control and Prevention	Prospective
#NOTREQ	Xi'an International Medical Center Hospital	Retrospective
Shanghai Science and Technology Commission	Shanghai Public Health Clinical Center	Prospective
#NOTREQ	Brii Biosciences Limited	Prospective
Project funded by Shanxi Provincial Key Laboratory for major infectious	Shanxi Provincial Center for Disease Control and Prevention	Retrospective
Project funded by Shanxi Provincial Key Laboratory for major infectious	Shanxi Provincial Center for Disease Control and Prevention	Prospective
Project funded by Shanxi Provincial Key Laboratory for major infectious	Shanxi Center for Disease Control and Prevention	Prospective
Self-funded	Yunnan Walvax Biotechnology Co., Ltd.	Prospective
Shijiazhuang Yiling Pharmaceutical Co., Ltd.	Hebei Province Chest Hospital	Retrospective

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Shijiazhuang Yiling Pharmaceutical Co., Ltd.	Hebei Province Chest Hospital	Retrospective
Shijiazhuang Yiling Pharmaceutical Co., Ltd.	Hebei Province Chest Hospital	Retrospective
self-financing	Shu Lan (Hang Zhou) Hospital	Prospective
#NOTREQ	Jiangsu Provincial Center for Disease Control and Prevention	Prospective
covid-19 Emergency project	Tianjin University of TCM	Prospective
Shijiazhuang Yiling Pharmaceutical Co.Ltd.	The Second Hospital of Hebei Medical University	Prospective
Sichuan Jishengtang Pharmaceutical Co. Ltd	Hubei Hospital of Traditional Chinese Medicine	Retrospective
#NOTREQ	Brii Biosciences Limited	Prospective
Translational Medical Center for Stem Cell Therapy of Shanghai	Shanghai Oriental Hospital	Retrospective
#NOTREQ	Sinovac Research and Development Co., Ltd.	Prospective

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#NOTREQ	Jiangsu Province Centers for Disease Control and Prevention	Prospective
None	Public Health Clinical Center of Chengdu	Prospective
#NOTREQ	Jiangsu Province Centers for Disease Control and Prevention	Prospective
Livzon Mabpharm Inc.	Guangdong Provincial Center for Disease Control and Prevention/ Gaozhou Municipal Center for Disease Control and Prevention	Retrospective
Livzon Mabpharm Inc.	Guangdong Provincial Center for Disease Control and Prevention/ Gaozhou Municipal Center for Disease Control and Prevention	Retrospective
Self-funded	The First Affiliated Hospital, Sun Yat-Sen University	Prospective
#NOTREQ	CanSino Biologics Inc.	Prospective
#NOTREQ	National Vaccine and Serum Institute, China	Prospective
#NOTREQ	China National Biotec Group Company Limited	Prospective
Shangnai Municipal Science and Technology Commission	shulan (Hangzhou) Hospital	Retrospective

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#NOTREQ	Sinovac Research and Development Co., Ltd.	Retrospective
China National Biotec Group Company Limited; The National Key	Shanghai Municipal Center for Disease Control and Prevention	Prospective
Wuhan Institute of Biological Products Co., Ltd.	Hubei Provincial Center for Disease Control and Prevention	Prospective
Project of Ministry of science and technology	Shijiazhuang People's Hospital	Retrospective
#NOTREQ	Qiang Hu	Retrospective
#NOTREQ	CanSino Biologics Inc.	Prospective
#NOTREQ	Jiangsu Province Centers for Disease Control and Prevention	Prospective
Self financing	Jiangsu Provincial Center For Disease Control and Prevention (Public Health Research Institute of Jiangsu Province)	Retrospective
Translational Medical Center for Stem Cell Therapy of Shanghai	Shanghai East Hospital	Retrospective
Guangdong Provincial Department of Science and Technology	Guangdong Provincial Hospital of Traditional Chinese Medicine	Retrospective

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#NOTREQ	Zhejiang Clover Biopharmaceuticals, Inc.	Prospective
#NOTREQ	Luye Pharma Group Ltd.	Retrospective
#NOTREQ	Sinovac Research and Development Co., Ltd.	Prospective
#NOTREQ	Anhui Zhifei Longcom Biologic Pharmacy Co., Ltd.	Retrospective
#NOTREQ	Shanghai Zerun Biotechnology Co.,Ltd	Retrospective
#NOTREQ	Shanghai Zerun Biotechnology Co.,Ltd	Retrospective
#NOTREQ	Sinovac Research and Development Co., Ltd.	Prospective
#NOTREQ	Shenzhen Kangtai Biological Products Co., LTD	Prospective
#NOTREQ	Shenzhen Kangtai Biological Products Co., LTD	Retrospective
#NOTREQ	Sinovac Research and Development Co., Ltd.	Prospective

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#NOTREQ	WestVac Biopharma Co., Ltd.	Retrospective
Ningbo Rong an Biological pharmaceutical Co., LTD	Hunan Provincial Center for Disease Control and Prevention	Prospective
Jiangsu Provincial Department of Science and Technology	Affiliated Hospital of Nanjing University of Chinese Medicine	Retrospective
State Administration of Traditional Chinese Medicine	State Administration of Traditional Chinese Medicine	Retrospective
National Administration of Traditional Chinese Medicine	Guangdong Provincial Hospital of Chinese Medicine	Retrospective
National Key R&D Program of China (No.2020YFC0841500);	China Academy of Chinese Medical Sciences; Jiangsu Privince Hospital of Chinese Medicine; Yangzhou Hospital of Traditional	Prospective
Self-funded	Walvax Biotechnology Co., Ltd.	Prospective
Self-funded	Shanxi Provincial Center for Disease Control and Prevention	Retrospective
Chongqing Key Laboratory of Ophthalmology (CSTC, 2008CA5003)	The First Affiliated Hospital of Chongqing Medical University	Prospective
Self-funded	Shulan (Hangzhou) Hospital	Retrospective

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Self-funded	Guangxi Center for Disease Control and Prevention	Prospective
Health Commission of Hubei Province	Hubei Provincial Hospital of Integrated Chinese and Western Medicine	Retrospective
#NOTREQ	Jieming QU	Prospective
#NOTREQ	Shanghai Public Health Clinical Center	Prospective
Shanghai University of TCM research program for 2019-ncov	Shanghai University of TCM	Prospective
#NOTREQ	Jiangsu Province Centers for Disease Control and Prevention	Prospective
Health Commission of Hubei Province	Hubei Provincial Hospital of Integrated Chinese and Western Medicine	Prospective
#NOTREQ	Jiangsu Province Centers for Disease Control and Prevention	Prospective
#NOTREQ	Sinovac Research and Development Co., Ltd.	Retrospective
#NOTREQ	China National Biotec Group Company Limited	Retrospective

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#NOTREQ	Peking Union Medical College Hospital	Retrospective
#NOTREQ	Sinovac Biotech Co., Ltd	Prospective
#NOTREQ	Anhui Zhifei Longcom Biologic Pharmacy Co., Ltd.	Retrospective
Hubei Provincial Health and Health Commission	Taihe Hospital of Shiyan City	Prospective
self-raised	Renmin Hospital of Wuhan University	Prospective
Ningbo Rong 'an biological pharmaceutical Co. LTD	Hunan Provincial Center for Disease Control and Prevention	Prospective
Chen Zhihai's funding for Changjiang Scholars	Beijing Ditan Hospital, Capital Medical University	Retrospective
The First Affiliated Hospital of Zhengzhou University	The First Affiliated Hospital of Zhengzhou University	Retrospective
Study group at their own expense	Shanghai Public Health Clinical Center	Retrospective
#NOTREQ	Anhui Zhifei Longcom Biologic Pharmacy Co., Ltd.	Retrospective

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Self-funded	Walvax Biotechnology Co., Ltd.	Prospective
#NOTREQ	Sinovac Biotech Co., Ltd	Prospective
sponsor	Sichuan Provincial Center for Disease Control and Prevention	Prospective
Sponsor	Shanghai Public Health Clinical Center	Retrospective
#NOTREQ	Sinovac Research and Development Co., Ltd.	Retrospective
#NOTREQ	Jiangsu Province Centers for Disease Control and Prevention	Prospective
#NOTREQ	Sinovac Research and Development Co., Ltd.	Prospective
#NOTREQ	Sinovac Research and Development Co., Ltd.	Retrospective
#NOTREQ	Innovent Biologics (Suzhou) Co. Ltd.	Prospective
Self-funded	Yunnan Walvax Biotechnology Co., Ltd.	Prospective

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#NOTREQ	Shanghai Junshi Bioscience Co., Ltd.	Prospective
#NOTREQ	Peking University	Prospective
#NOTREQ	Peking University	Prospective
NA	Hubei Provincial Hospital of Traditional Chinese Medicine	Retrospective
Self-funded	Sichuan Center for Disease Control and Prevention	Retrospective
Beijing Ditan Hospital, Capital Medical University	Beijing Ditan Hospital, Capital Medical University	Retrospective
Self-funded	Yunnan Walvax Biotechnology Co., Ltd.	Retrospective
Postgraduate Research & Practice Innovation Program of	Yangzhou University	Retrospective
Sponosor	Jiangsu Provincial Center for Disease Control and Prevention	Prospective
#NOTREQ	Jiangsu Province Centers for Disease Control and Prevention	Prospective

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#NOTREQ	Frontier Biotechnologies Inc.	Prospective
#NOTREQ	Huashan Hospital	Retrospective
#NOTREQ	Sinocelltech Ltd.	Prospective
#NOTREQ	AstraZeneca	Prospective
Sichuan Health Care Commission Medical Major Innovation	West China Hospital of Sichuan University	Retrospective
Changde City Science and Technology Bure	The first people's hospital of changde city	Retrospective

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Ethical Approval Status (Approved = 1 or 0 = 0)	Name of Ethic Committee	Ethic Committee Approval No.
1	Medical Ethics Committee of Wuhan Infectious Disease Hospital	KY-2020-02.01
#NOTREQ	#NOTREQ	#NOTREQ
#NOTREQ	#NOTREQ	#NOTREQ
1	The Ethic Committee of Chongqing Public Health Medical Center	2020-003-02-KY
1	The Institution Review Board of Chongqing Public Health Medical Center	2020-002-01-KY
#NOTREQ	#NOTREQ	#NOTREQ
1	Ethics Committee of Dongzhimen Hospital Affiliated to Beijing University of Chinese Medicine	DZMEC-KY-2020-09
#NOTREQ	#NOTREQ	#NOTREQ
#NOTREQ	#NOTREQ	#NOTREQ

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#NOTREQ	#NOTREQ	#NOTREQ
#NOTREQ	#NOTREQ	#NOTREQ
1	Zhongshan Hospital Affiliated to Dalian University ethics committee	2020003
1	Ethic committee of Hebei Yiling Hospital	2020LCKY-002
1	Ethics Committee of Hebei Yiling Hospital/ Ethics Committee of Renmin Hospital of Wuhan University	2020LCKY-001/ WDRY2020-K027
1	Medical ethics committee of Wuhan 1st Hospital	武卫一院伦审[2020]1号
1	Clinical trial ethics committee of Hubei integrated hospital of traditional Chinese and Western Medicine	[2020]004
1	Clinical trial ethics committee of Hubei integrated hospital of traditional Chinese and Western Medicine	[2020]002
1	Ethics Committee of Hubei Provincial Hospital of Integrated Chinese and Western Medicine	2020007
1	Ethics Committee of Hubei Provincial Hospital of Integrated Chinese and Western Medicine	2020008

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1	Ethics Committee of Hubei Provincial Hospital of Integrated Chinese and Western Medicine	2020006
1	Ethics Committee of Hubei Provincial Hospital of Integrated Chinese and Western Medicine	2020009
1	Ethics Committee of Hubei Provincial Hospital of Integrated Chinese and Western Medicine	2020010
#NOTREQ	#NOTREQ	#NOTREQ
#NOTREQ	#NOTREQ	#NOTREQ
#NOTREQ	#NOTREQ	#NOTREQ
1	The Second Xiangya Hospital of Central South University Ethics Committee / Medical Ethics Committee of Changsha 1st Hospital	KX2020-002; 2020-003
1	Medical Ethics Committee of Hangzhou Xixi Hospital	2020年（科）伦审第12号
1	Medical Ethics Committee of Hangzhou Xixi Hospital	2020年（科）伦审第13号
1	The Ethic Committee of Tongji Hospital, Tongji Medical College, Huazhong University of Science and Technology	TJ-C20200102

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1	The ethic committee of Zhongnan Hospital of Wuhan University	临研伦【2020011】
1	Clinical Research Ethics Committee of The First Hospital Affiliated to Zhejiang University's Medical School	（2020）IIT会审第（7）号
1	Clinical Research Ethics Committee of the First Affiliated Hospital,College of Medicine,Zhejiang University	(2020)IIT会审第（6）号
1	Sichuan Regional Ethical Review Committee of Traditional Chinese Medicine	2020QKL－004
1	Clinical Research Ethics Committee of Wuhan University Renmin Hospital	WDRY2020-K012
1	EC of Xiangyang 1st People's Hospital	2020GCP008
#NOTREQ	#NOTREQ	#NOTREQ
#NOTREQ	#NOTREQ	#NOTREQ
1	Ethics Committee of Xiangyang First People's Hospital	2020GCP010
1	Clinical Research Ethics Committee of the First Affiliated Hospital of Medical College of Zhejiang University	（2020）IIT快审第（26）号

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1	Medical ethics committee of Department of Hematology, Tongji Hospital, Tongji Medical College, Huazhong University of Science and Technology	TJ-C20200115
1	Medical Ethics Committee of Tianjin University of TCM	TJUTCM-EC20200001
1	Medical Ethics Committee of Hubei Provincial Hospital of TCM	HBZY2020-C01-01
1	Medical Ethics Committee of Hubei Provincial Hospital of TCM	HBZY2020-C01-01
1	The Clinical Research Ethic Committee of the First Affiliated Hospital of Zhejiang University School of Medicine	（2020）IIT会审第（8）号，（2020）IIT快审第（6）号
1	Ethical Committed of Tongji Hospital, Tongji Medical College, Huazhong University of Science and Technology	TJ-C20200113
#NOTREQ	#NOTREQ	#NOTREQ
1	Ethics Committee on Stem Cell of the First Affiliated Hospital of Zhejiang University School of Medicine	[2020]GXB伦审001号
1	Clinical Trial Ethics Committee, West China Hospital of Sichuan University	2020年临床试验（西药）审（10）号
1	Medical Ethics Committee of Wuhan Pulmonary Hospital	武肺伦理(2020)年第（1）号

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1	Ethics Committee in Central Theater General Hospital of PLA	【2020】009-3
1	Peking University First hospital Human Research Ethics committee	2020科研031
1	Medical Ethics Committee of the Fifth Affiliated Hospital Sun Yat-Sen University	中大五院【2020】伦字第（K15-1）号
1	Ethics Committee of Tongji Hospital, Tongji Medical College, Huazhong University of Science and Technology	TJ-C20200116
1	The First Affiliated Hospital of Anhui University of Traditional Chinese Medicine	2020HSAH-01
1	Institutional Review Board, Tongji Hospital, Tongji Medical College, Huazhong University of Science and Technology	TJ-C20200124
1	Ethics Committee of Clinical Research, Renmin Hospital of Wuhan University	WDRY2020-K018
1	The Ethics Review Committee of Institute of Blood Transfusion, Chinese Academy of Medical Sciences	202002
1	Ethic Committee of the Second Affiliated Hospital of Chongqing Medical University	2020年伦审(2)号
1	The Ethic Committee of the Second Affiliated Hospital of Chongqing Medical University	2020年伦审(1)号

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1	Ethics Committee of the First Affiliated Hospital of Chongqing Medical University	2020年科伦药审（20200101）号
1	The First Affiliated Hospital of Chongqing Medical University ethics committee	20200401
1	Ethics Review Committee of Institute of Basic Research in Clinical Medicine, China Academy of Chinese Medical Sciences	P20001/PJ01
1	The First Affiliated Hospital of University of science and technology of China (Anhui Provincial Hospital) Ethics committee	2020-XG（H）-008
1	The Ethics Committee of Zhongnan Hospital of Wuhan University	临研伦[2020016]
1	Hospital Ethics Committee of Enze Hospital of Taizhou Enze Medical Center (Group)	无
1	Ethics Committee in Clinical Research of the First Affiliated of Wenzhou Medical University	2020第（001）号
1	Ethics Committee of Huangshi Hospital of Traditional Chinese Medicine	HSZY-PJ-2020-001-01
1	Medical Ethics Committee Union Hospital affiliated to Tongji Medical College of Huazhong University of Science and Technology	【2020】伦理字（0012）号
1	Medical Ethics Committee Union Hospital affiliated to Tongji Medical College of Huazhong University of Science and Technology	[2020]论审字（012）号

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0	#BLANK	#BLANK
1	Institutional Review Board of the First Affiliated Hospital of He'nan University of Chinese Medicine	2020HL-016
0	#BLANK	#BLANK
#NOTREQ	#NOTREQ	#NOTREQ
1	Ethics Committee of Clinical Research, Renmin Hospital of Wuhan University	WDRY2020-K037
1	Medical Ethics Committee of Wuhan Infectious Disease Hospital	KY-2020-10.02
1	Shanghai Public Health Clinical Center Ethics Committee	YJ-2020-E004-01
1	Medical Ethics Committee of Nanjing Second Hospital	2020-LS-ky006
1	Ethic committee of Jingzhou Central Hospital	2020-004-01
#NOTREQ	#NOTREQ	#NOTREQ

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#NOTREQ	#NOTREQ	#NOTREQ
1	Ethic committee of Jingzhou Central Hospital	2020-003-01
1	Ethics Committee of Scientific Research and Clinical Trials of the First Affiliated Hospital of Zhengzhou University	2020-KY-056
1	Ethics Committee of Zhongshan Hospital, Fudan University	B2020-030
1	People's Hospital of Guangshan County EMC	光伦字[2020]001号
0	#BLANK	#BLANK
1	Ethics Committee of Beijing Youan Hospital, Capital Medical University	京佑科伦字[2020]017号
1	Ruijin Hospital Ethic Committee, Shanghai Jiaotong University School of Medicine	（2020）临伦审第（29）号/KY2020-29
1	Ethics Committee of Huangshi Hospital of Traditional Chinese Medicine	HSZYYJ-2020-002-01
#NOTREQ	#NOTREQ	#NOTREQ

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#NOTREQ	#NOTREQ	#NOTREQ
1	Peking University Third Hospital Medical Science Research Ethics Committee	（2020）医伦审第（041-05）号
1	Peking University Third Hospital Medical Science Research Ethics Committee	（2020）医伦审第（042-05）号
1	Human Research Ethics Committee of HwaMei Hospital, University of Chinese Academy of Sciences	PJ-NBEY-KY-2020-006-01
0	#BLANK	#BLANK
0	#BLANK	#BLANK
0	#BLANK	#BLANK
1	Sichuan Regional Ethical Review Committee of Traditional Chinese Medicine	2020QKL一005
1	Ethical Committee of Tongji Hospital, Tongji Medical College,Huazhong University of Science and Technology	TJ-C20200127
1	Ethic Committee of Qilu Hospital of Shandong University	(科）伦审第2020 002号

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0	#BLANK	#BLANK
1	Medical Ethics Committee, Zhongnan Hospital of Wuhan University	2020019
0	#BLANK	#BLANK
1	Ethical Committee of Beijing You-An Hospital	京佑科伦字[2020]007号
1	IRB of the First Affiliated Hospital of Nanchang University	【2020】临伦审第001号
1	Medical Ethics Committee of Zhongshan Hospital Affiliated to Xiamen University	xmzsyyky伦理第（2020004）号
1	Ethics Committee of the First Affiliated Hospital of Guangzhou Medical University	医研伦审2020第54号
1	Huangshi city hospital of TCM ethic committee, Hubei province	HSZYYJ-2020-003-01
1	Ethics committee of Heilongjiang Provincial hospital	003JJ02
1	The First Affiliated Hospital of University of science and technology of China (Anhui Provincial Hospital) Ethics committee	2020-XG(H)-016

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1	Sichuan Regional Ethical Review Committee of Traditional Chinese Medicine	2020QKL一006
0	#BLANK	#BLANK
1	Clinical Trial Ethics Committee of the First Affiliated Hospital of Guangzhou Medical University	医研伦审2020第48号
1	Medical Ethics Committee of Wuhan Infectious Disease Hospital	KY-2020-17.02
1	Medical ethics committee of the Fourth People's Hospital of Nanning	202002
#NOTREQ	#NOTREQ	#NOTREQ
1	Ethics Committee of Beijing Chaoyang Hospital, Capital Medical University	2020-科-21-1
1	Wuhan Fourth Hospital ethic committee	202001005
1	IRB of the first affiliated hospital of Anhui Medical University	安医一附院伦申-快-PJ2020-03-19
1	China Ethics Committee of Registering Clinical Trials	ChiECRCT20200035

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1	Institutional Review Board of Qingyuan People's Hospital	IRB-2020-002
1	Ethics Committee of the First Affiliated Hospital of Guangzhou Medical University	医研伦审2020第45号
1	Medical ethic committee of Tongde hospital of ZheJiang Province	浙同德伦理审字第【2020】001号
1	Peking University Third Hospital Medical Science Research Ethics Committee	（2020）医伦审第（033-03）号
#NOTREQ	#NOTREQ	#NOTREQ
1	Medical ethics committee of Zhongshan Hospital Affiliated to Xiamen University	xmzsyyky伦理第（2020004）号
1	Medical Ethics Committee of the First Affiliated Hospital of Guangzhou Medical University	医研伦审2020第55号
1	Clinical research ethics committee of Renmin Hospital of Wuhan University	WDRY2020-K047
1	Clinical Research Ethics Committee of the First Affiliated Hospital of Nanchang University	ndyfy(2020)医研伦审第（40）号
1	Ethic Oommittee of the First Hospital of Shanxi Medical University	Q02

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1	Medical ethics committee of PLA Navy General Hospital	[2018]0830
1	Biomedical research ethics committee of Shanghai Changzheng Hospital	2020SL004
0	#BLANK	#BLANK
1	Ethics Committee of Stem Cell Clinical Research Institute of the First Affiliated Hospital of Nanchang University	[2020]06号
1	Medical Ethics Committee of Shanghai Public Health Clinical Center	YJ-2020-E007-01
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1	Ethics Committee of Nanhua Hospital Affiliated to Nanhua University	2020-001-01
1	Medical Research Ethics Committee of the First Affiliated Hospital of Nanchang University	(2020)医研伦审第（41）号
1	Medical Ethics Committee of Jingzhou First People's Hospital	Y20200209
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#NOTREQ	#NOTREQ	#NOTREQ
#NOTREQ	#NOTREQ	#NOTREQ
1	Biomedical Ethics Committee of West China Hospital, Sichuan University	2020年审（163）号
1	Medical Research Ethics Committee of the First Affiliated Hospital of Nanchang University	（2020）医研伦审第（43）号
1	Medical Ethics Committee of Ganzhou Fifth People's Hospital	医伦审（2020）1号
1	Medical ethics committee of The First People’s Hospital of Jiangxia District	(2020)伦审批第013号

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1	Ethic Committee of Yueyang Hospital of Integrated Traditional Chinese and Western Medicine, Shanghai University of Traditional Chinese Medicine, Shanghai, China	2020-003
1	Medical Ethics Commitee, Zhongnan Hospital of Wuhan University	科伦【2020040】、科伦【2020040-1】
1	IRB of Shuguang Hospital affiliated with Shanghai University of TCM	2020-796-03-01
1	Shanghai Changzheng Hospital	2020SL002
1	Biomedical Research Ethics Committee，Ganzi Hospital of West China Hospital, Sichuan University	GZZYY-2020-02-10
1	Ethics Committee of Wuxi Fifth People's Hospital	2020-003-1
1	The Ethics Committee of First Affiliated Hospital of Harbin Medical University	IRB-AF/SC-04/01.0
1	Ethics Review Committee of China Academy of Chinese Medical Sciences	CACMS-IRB2020-002-1
1	The Ethical Committee of Tongji Hospital of Tongji Medical College; Huazhong Science and Technology University	TJ-C20200108
1	Biomedical ethics committee of West China Hospital of Sichuan University	2020年 审（194）号

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0	#BLANK	#BLANK
1	Ethics committee of the second affiliated hospital of xi 'an medical university	X2Y202001
#NOTREQ	#NOTREQ	#NOTREQ
1	Ethics commitee of the first people's hospital of Jiangxia district, Wuhan	2020-0003
1	Medical Ethics Committee of Jingzhou First People's Hospital	Y20200211
1	Medical Ethics Committee of Wuhan Infectious Disease Hospital	KY-2020-26.02
1	Ethics Committee of Heilongjiang Social Rehabilitation Hospital	[2020]院伦审临医字第（001）号
1	Ethics review form for branch medical research and clinical technology application, ethics committee of First Affiliated Hospital of Fujian Medical University	闽医大附一伦理炎【2020】038
1	Clinical Trial Center of Zhongnan Hospital Affiliated to Wuhan University	20200228
1	Ethics Committee of the First Affiliated Hospital of Fujian Medical University	闽医大附一伦理医研[2020]039号

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1	EC of The Fourth People's Hospital of Wuhan	SY2020-027
1	Biomedical ethics committe of Dalian Medical University	2020004
1	IRB of Shuguang Hospital affiliated to Shanghai University of T.C.M.	20207970401
1	Ethic Committee of Maoming People's Hospital	2020013
1	Ethics Committee Longhua Hospital, Shanghai University of Traditional Chinese Medicine	2020LCSY008
1	Ethics Committee of Jiangsu Province Hospital on Integration of Chinese and Western Medicine	2020LWKY011
1	Ethics Committee of Jiangsu Province Hospital on Integration of Chinese and Western Medicine	2020LWKY009
1	Tongji Hospital, Tongji Medical College, Huazhong University of Science and Technology Institutional Review Board Approval	IRB ID:TJ-C20200109
1	Medical Ethics Committee， Zhongnan Hospital of Wuhan University	2020052
1	Ethics committee of the first affiliated hospital of guangzhou university of Chinese medicine	NO.ZYYECK【2020】017

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1	Medical research ethics committee of Shiyan Taihe hospital	202001
#NOTREQ	#NOTREQ	#NOTREQ
1	Ethical Committee of The First Hospital of Harbin Medical University	IRB-AF/SC-04/01.0
1	Ethics Committee of the Second Affiliated Hospital of Wenzhou Medical University	LCKY2020-11
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1	Ethics Committee of Wuhan Red Cross Hospital	2020009
1	Medical ethics committee of Tongji Hospital Affiliated to Tongji Medical College of Huazhong University of science and technology	TJ-IRB20200318
1	Ethics Committee of Hubei Hospital of traditional Chinese Medicine	HBZY2020-A04-01
0	#BLANK	#BLANK
1	The Ethic Committee of Shanghai General Hospital	院伦审[2019]15

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1	Ethics Committee of Scientific Research and Clinical Trials of the First Affiliated Hospital of Zhengzhou University	2020-KY-060
#NOTREQ	#NOTREQ	#NOTREQ
1	Medical ethics committee of Liaocheng People's Hospital	2020004
#NOTREQ	#NOTREQ	#NOTREQ
1	Ethics Committee of Union Hospital affiliated to Tongji Medical College of Huazhong University of Science and Technology	[2020]伦审字（0083）号
1	Ethics Committee of research study of the Third People's Hospital of Shenzhen	深圳三院伦审科研字[2020-008-02]号
1	The Ethics Review Committee of Institute of Blood Transfusion, Chinese Academy of Medical Sciences	202003
1	Medical Ethics Committee of Xiangya Hospital Central South University	202003025
1	Medical ethics committee of the first people's Hospital of Jiangxia District	2020伦审批第14号
1	Medical Ethics Committee, Zhongnan Hospital of Wuhan University	2020019

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1	Medical Ethics Committee of Hubei Provincial Hospital of TCM	HBZY2020-C01-01
0	#BLANK	#BLANK
1	Professional ethics committee for clinical research of The First Affiliated Hospital of Wenzhou Medical University	KY2020-003
1	Biomedical research ethics committee of Shanghai Changzheng Hospital	2020SL005
1	Ethic Committee of Maoming People's Hospital	2020012
#NOTREQ	#NOTREQ	#NOTREQ
1	Ethics Committee of Scientific Research Project Review of the First Affiliated Hospital of Guangzhou Medical University	医研伦审2020第59号
1	Human Research Ethics Committee of HwaMei Hospital, University of Chinese Academy of Sciences	PJ-NBEY-KY-2020-022-01
1	Human Research Ethics Committee of HwaMei Hospital, University of Chinese Academy of Sciences	PJ-NBEY-KY-2020-021-01
1	Ethics Committee of Foshan First People's Hospital	伦审研[2020]第3号

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1	IEC of Affiliated Hospital of Shaanxi University of Traditional Chinese Medicine	SZFYIEC-PJ-2020年第3号
1	Ethics Committee of Clinical Research, Renmin Hospital of Wuhan University	WDRY2020-K109
1	Medical Research Ethics Committee of the First Affiliated Hospital of Nanchang University	(2020)医研伦审第(47)号
1	Clinical Trial Ethics Committee of the First Affiliated Hospital of Guangzhou Medical University	EC-2020-016(YW)-02
1	Medical research ethics committee of the Second Affiliated Hospital of Nanchang University	(2020)医研伦审第（01）号
#NOTREQ	#NOTREQ	#NOTREQ
1	Ethical committee of Guangdong provincial hospital of Chinese medicine	AF2020-057-01
1	Medical Ethics Committee of Xiyuan Hospital, Chinese Academy of Traditional Chinese Medicine	2020XLA022-1
1	Ethic commitee of Ningbo Kangning Hospital	NBKNYY-2929-LC-08
1	Ethics Committee of the Third Xiangya Hospital of Central South University	快20040

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0	#BLANK	#BLANK
#NOTREQ	#NOTREQ	#NOTREQ
1	Ethics committee of Beijing You'an Hospital, Capital Medical University	京佑科伦字【2020】006号
#NOTREQ	#NOTREQ	#NOTREQ
#NOTREQ	#NOTREQ	#NOTREQ
#NOTREQ	#NOTREQ	#NOTREQ
1	Medical ethics committee of the Third Affiliated Hospital of Sun Yat-sen University	[2020]02-037
1	Institutional Review Board of The First Teaching Hospital of Tianjin University of Traditional Chinese Medicine	TYLL2020[K]字009
#NOTREQ	#NOTREQ	#NOTREQ
1	Tongji Hospital, Tongji Medical College, Huazhong University of Science and Technology Institutional Review Board Approval	TJ-IRB20200389

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1	Medical ethics committee of The Fifth Affiliated Hospital of Sun Yat-Sen University	中大五院[2020】伦字第K16-1号
#NOTREQ	#NOTREQ	#NOTREQ
#NOTREQ	#NOTREQ	#NOTREQ
0	#BLANK	#BLANK
1	Ethics Committee of Beijing University of Chinese Medicine	2020BZHYLL0111
#NOTREQ	#NOTREQ	#NOTREQ
1	Medical ethics committee of huzhou central hospital	20200214-01
1	Medical Ethics Commission of Henan Provincial Center for Disease Control and Prevention	2020-YM-002-02
1	Ethics committee of the First Affiliated Hospital with Nanjing Medical University	2020-SR-171
1	Guangzhou Eighth People's Hospital Ethic Committee	科 202010143

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#NOTREQ	#NOTREQ	#NOTREQ
1	Ethics Review Committee of China Academy of Chinese Medical Sciences	CACMS-IRB2020-004
1	Ethics Committee of Beijing University of Chinese Medicine	2020BZHYLL0113
1	Wuhan Huoshenshan Hospital Medical Ethics Committee	HSSLL018
1	Shanghai Public Health Clinical Center Ethics Committee	YJ-2020-S073-01
#NOTREQ	#NOTREQ	#NOTREQ
1	Medical Ethics Committee of Hubei Provincial Hospital of TCM	HBZY2020-C21-01
1	Medical ethics committee of Wuhan 1st Hospital	武卫一院伦审【2020】9号
1	Ethics Committee of Hubei Provincial Hospital of TCM	HBZY2020-C27-01
1	Medical Ethics Committee of Mudanjiang Kang'an Hospital	医伦审202004号

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#NOTREQ	#NOTREQ	#NOTREQ
1	Scientific Research Ethics Committee of Beijing University of Chinese Medicine Third Affiliated Hospital	ECBJZYYKJFZZJSL-ZYDSY2020-01
1	Biomedical ethics committee, west China hospital, sichuan university	2020(206)
1	Ethics Committee of Yueyang Hospital of Integrated Traditional Chinese and Western Medicine, Shanghai University of Traditional Chinese Medicine	2020-002-01
1	Xiaogan Chinese medicine hospital ethics committee	HBXGZYY-2020-01
1	The Ethics Committee of Union Hospital affiliated to Tongji Medical College of Huazhong University of Science and Technology	N/A
1	Medical Ethics Commission of Henan Provincial Center for Disease Control and Prevention	2020-YM-001-02
1	Ethics Committee of Shanghai Public Health Clinical Center	2020-S063-01
1	The Ethical Committee of the Research Project Review of the First Affiliated Hospital of Guangzhou Medical University	2020-87
1	Medical Ethics Committee of Hubei Provincial Hospital of TCM	HBZY2020-C21-01

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1	Ethics Committee of the First Affiliated Hospital of Guangdong Pharmaceutical University	医伦审【2020】第（62）号
1	Ethics committee of Beijing Ditan Hospital affiliated to Capital Medical University	京地伦药字（2020）第（014）-02号
#NOTREQ	#NOTREQ	#NOTREQ
#NOTREQ	#NOTREQ	#NOTREQ
1	Medical Ethics Committee of Hongqi Hospital Affiliated to Mudangjiang Medical University	202014
1	Ethics Committee of Wuhan Third Hospital	武三医院KY2020-050
1	the Institutional Ethical Review Board of the First Hospital of China Medical University	2020-14-2
1	Ethics committee office of The First Affiliated Hospital of Wenzhou Medical University	（2020）第（002）
#NOTREQ	#NOTREQ	#NOTREQ
1	The Ethics Committee of the First Affiliated Hospital of Nanjing Medical University	2020-SR-149

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1	The ethic committee of Guangzhou Women and Children's Medical Center	202028500
1	The Medical Ethics Committee of Xiangyang General Hospital	XYSZXYY-LL-PJ-2020-001
1	Ethics Committee of Yueyang Hospital of Integrated Traditional Chinese and Western Medicine, Shanghai University of Traditional Chinese Medicine	2020-002-01
1	Shanghai Public Health Clinical Center Ethics Committee	[2020]YJ-2020-S065-02
#NOTREQ	#NOTREQ	#NOTREQ
1	Medical Ethics Committee of Children's Hospital Affiliated to Zhejiang University School of Medicine	2020-IEC-004
#NOTREQ	#NOTREQ	#NOTREQ
1	Ethics Committee of the First Affiliated Hospital of Bengbu Medical College	BYYFY-2020KY10
1	Shulan (Hangzhou) Hospital Ethics Committee for Clinical Trials	2020伦审第（18）号、GXIRB2020-0046
#NOTREQ	#NOTREQ	#NOTREQ

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#NOTREQ	#NOTREQ	#NOTREQ
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#NOTREQ	#NOTREQ	#NOTREQ
1	Ethics Committee of Hubei Hospital of Traditional Chinese Medicine	HBZY2020-C26-01
1	Shanghai Public Health Clinical Center Ethics Committee	YJ-2020-S124-02
#NOTREQ	#NOTREQ	#NOTREQ
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#NOTREQ	#NOTREQ	#NOTREQ

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1	The Ethical Review Committee of Institute of Pathogen Biology, Chinese Academy of Medical Sciences	IPB-2020-22
1	Ethics Committee of Jiangsu Provincial Center for Disease Prevention and Control	JSJK2020-A039-01
1	Shanghai Ethics Committee for Clinical Research	SECCR/2020-20-03
#NOTREQ	#NOTREQ	#NOTREQ
1	IRB of The Third Affiliated Hospital of Beijing University of Traditional Chinese Medicine	BZYSY-2020KYKTPJ-07
1	IRB of The Third Affiliated Hospital of Beijing University of Traditional Chinese Medicine	BZYSY-2020KYKTPJ-06
#NOTREQ	#NOTREQ	#NOTREQ
#NOTREQ	#NOTREQ	#NOTREQ
1	Research Ethics Committee of the Third Affiliated Hospital of Beijing University of Chinese Medicine	BZYSY-2020KYKTPJ-06
1	Ethic Committee of Jiangsu Provincial Center for Disease Control and Prevention	JSJK2020-A057-02

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#NOTREQ	#NOTREQ	#NOTREQ
1	Shulan (Hangzhou) Hospital Ethics Committee for Clinical Trials	2020伦审第（31）号
1	Ethics Committee of Xiangzhou District People's Hospital	202014
1	Ethic Committee of Jiangsu Provincial Center for Disease Control and Prevention	JSJK2020-A058-02
#NOTREQ	#NOTREQ	#NOTREQ
1	Ethics Committee of Jiangsu Provincial Center for Disease Prevention and Control	JSJK2020-A061-01
#NOTREQ	#NOTREQ	#NOTREQ
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#NOTREQ	#NOTREQ	#NOTREQ

For peer review only

1	Ethics Committee of Jiangsu Provincial Center for Disease Control and Prevention	JSJK2020-A063-02
1	Ethics Committee of Hu'nan Provincial Center for Disease Control and Prevention	湘疾控IRB2020040
#NOTREQ	#NOTREQ	#NOTREQ
1	Shanghai Public Health Clinical Center Ethics Committee	2020-S206-01
#NOTREQ	#NOTREQ	#NOTREQ
1	Ethics Committee of Shanxi Provincial Center for Disease Control and Prevention	SXCDCIRBPJ2020056001
1	Ethics Committee of Shanxi Provincial Center for Disease Control and Prevention	SXCDCIRBPJ2020056001
1	Ethics Committee of Shanxi Center for Disease Control and Prevention	SXCDCIRBPJ2020056001
1	Guangxi IRB	GXIRB2020-0066-1
1	Ethics Committee of Hebei Province Chest Hospital	2021002

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1	Ethics Committee of Hebei Province Chest Hospital	2021001
1	Ethics Committee of Hebei Province Chest Hospital	2021003
1	Shulan (Hangzhou) Hospital Ethics Committee for Clinical Trials	2021伦审第（1）号
1	#NOTREQ	#NOTREQ
1	Medical Ethics Committee of Tianjin University of TCM	TJUTCM-EC20210001
1	Research Ethics Committee of the Second Hospital of Hebei Medical University	2021-R013-01
1	Ethics Committee of Hubei Hospital of Traditional Chinese Medicine	HBZY2020-C27-01
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1	Ethics Committee of Shanghai East Hospital	2020研审第096号
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1	Medical Ethics Committee of Chengdu Public Health Clinical Medical Center	PJ-K2021-01-01
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1	The Ethics Committee for Clinical Trials of Vaccines, Guangdong Provincial Center for Disease Control and Prevention	2021V002-F
1	The Ethics Committee for Clinical Trials of Vaccines, Guangdong Provincial Center for Disease Control and Prevention	2021V001-F
1	Ethics Committee of the First Affiliated Hospital, Sun Yat-Sen University	[2021]254
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1	Shulan (Hangzhou) Hospital Clinical Trial Ethics Committee	2021伦审第（3）号

For peer review only

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1	SHANGHAI MUNICIPALCENTER FOR DISEASE CONTROL AND PREVENTION ETHICAL REVIEW COMMITTEE FOR CLINICAL TRIAL	2021-7
1	Ethics Committee of Hubei Provincial Center for Disease Control and Prevention (Hubei Academy of Preventive Medicine)	2021-008-02
1	Medical Ethics Committee of Tianjin University of Traditional Chinese Medicine	TJUTCM-EC20210001
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1	Ethics Committee of Jiangsu Provincial Center for Disease Prevention and Control	JSJK2021-A017-01
1	Ethics Committee of Shanghai East Hospital	2021研审第055号
1	Ethics Committee of Guangdong Provincial Hospital of Traditional Chinese Medicine	BF2021-113-01

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1	Ethics Committee of Hu'nan Center for Disease Control and Prevention	湘疾控 IRB-PJ2021013
1	Ethics Committee of Affiliated Hospital of Nanjing University of Chinese Medicine	2020NL-005-03
1	Ethical Committee of Affiliated Hospital of Nanjing University of Chinese Medicine (Jiangsu Province Hospital of Chinese Medicine)	2021NL-123-02
1	Institutional Ethics Committee of Guangdong Provincial Hospital of Chinese Medicine	ZE2021-049-01
1	Ethics Review Committee of China Academy of Chinese Medical Sciences	CACMS-IRB2021-003
1	Yunnan CDC IRB	批件2021-4号
1	Ethics Review Committee of Shanxi Provincial Center for Disease Control and Prevention	SXCDCIRBYJ2021062002
1	Ethics Committee of the First Affiliated Hospital of Chongqing Medical University	20214601
1	Clinical Trial Ethics Committee of Shulan (Hangzhou) Hospital	2021伦审第（20）号

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1	Guangxi IRB	GXIRB2021-0026
1	Ethics Committee of Hubei Provincial Hospital of Integrated Chinese and Western Medicine	2020伦审第（研019）号
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1	Huangshi Hospital of TCM Ethic Committee	HSZYPJ-2021-001-01
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1	Ethics Committee of Hubei Provincial Hospital of Integrated Chinese and Western Medicine	2020伦审第（研020）号
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For peer review only

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1	Medical Research Ethics Committee of Taihe Hospital, Shiyan City	科研会审（202015）号
1	Clinical Research Ethics Committee of Renmin Hospital of Wuhan University	WDRY2021-K122
1	Ethics Committee of Hunan Center for Disease Control and Prevention	湘疾控 IRB-YJ2021028
1	Ethics Committee of Beijing Ditan Hospital, Capital Medical University	2021-033-02
1	The Ethics Committee of Scientific Research and Clinical Trial, The First Affiliated Hospital of Zhengzhou University	L2021-Y266-002
1	Medical Ethics Committee of Shanghai Public Health Clinical Center	YJ-2020-S123-02
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1	Guangxi Institutional Review Board	GXIRB2021-0042-1
#NOTREQ	#NOTREQ	#NOTREQ
1	Ethics Committee of Sichuan Provincial Center for Disease Control and Prevention	SC-0820212401
1	Medical Ethics Committee of Shanghai Public Health Clinical Center	2021-E020-02号
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1	Ethics Committee of Vaccine Clinical Trial, Yunnan Center for Disease Control and Prevention	批件2021-3号

For peer review only

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1	Ethics Committee of Hubei Provincial Hospital of TCM	HBZY2020-C27-01
1	Vaccine Clinical Trial Ethics Committee of Sichuan Center for Disease Control and Prevention	SC-0820212101
1	Ethics Committee of Beijing Ditan Hospital, Capital Medical University	京地伦科字〔2021〕第（024）-02号
1	YNIRB	批件2021-15号
1	Ethics Committee, School of Nursing, Yangzhou University	YZUHL2021017
1	Ethics Committee of Jiangsu Provincial Center for Disease Control and Prevention	JSJK2021-A034-02
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1	Medical Ethics Committee of Changde First People's Hospital	2021-077-01

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Ethic Committee Approval Date	Multicenter Flag (Yes = 1: if Number of Centers Declared by Investigator >1; No = 0)	Number of Centers Declared by Investigator
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For peer review only

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For peer review only

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For peer review only

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For peer review only

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2020/2/10	1	4
2020/2/11	0	1
2020/2/12	0	1
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For peer review only

2020/2/11	1	4
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For peer review only

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For peer review only

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For peer review only

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For peer review only

2013/8/26	1	2
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For peer review only

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For peer review only

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For peer review only

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For peer review only

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For peer review only

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For peer review only

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For peer review only

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For peer review only

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For peer review only

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Center name
Wuhan Jinyintan Hospital (Wuhan Infectious Diseases Hospital)
Medical ICU,Peking Union Medical College Hospital
The Fifth Medical Center, General Hospital of PLA
Chongqing Public Health Medical Center
Chongqing Public Health Medical Center
Shanghai Public Health Clinical Center
Hubei Provincial Hospital of Integrated Chinese and Western Medicine
Medical Center for Digestive Diseases, The Second Affiliated Hospital of Nanjing Medical University
Guangzhou Eighth People's Hospital

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Jin Yin-tan hospital

Bin Cao

Affiliated Zhongshan Hospital of Dalian University
Zhongnan Hospital of Wuhan University

Ninth Hospital of Wuhan
Shijiazhuang Fifth Hospital
Beijing YouAn Hospital
Henan provincial hospital for infectious diseases
The Second Affiliated Hospital of Chongqing Medical University
The No. 9 Hospital of Wuhan
The Central Hospital of EZhou
Fuyang Second People's Hospital
People's Hospital of Guizhou
The Infectious Disease hospital of Harbin

Wuhan 1st Hospital

Hubei integrated hospital of traditional Chinese and Western Medicine

Hubei integrated traditional Chinese and Western Medicine Hospital
WUHAN HOSPITAL OF TCM

Xinhua affiliated hospital, Hubei University of Chinese Medicine
Hubei Provincial Hospital of Integrated Chinese and Western Medicine

Xinhua affiliated hospital, Hubei University of Chinese Medicine
Hubei Provincial Hospital of Integrated Chinese and Western Medicine

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Xinhua affiliated hospital, Hubei University of Chinese Medicine
Hubei Provincial Hospital of Integrated Chinese and Western Medicine

Xinhua affiliated hospital, Hubei University of Chinese Medicine
Hubei Provincial Hospital of Integrated Chinese and Western Medicine

Xinhua affiliated hospital, Hubei University of Chinese Medicine
Hubei Provincial Hospital of Integrated Chinese and Western Medicine

Department and Institute of Infectious Disease

Department and Institute of Infectious Disease

Department and Institute of Infectious Disease

The First Hospital of Changsha
The Second Xiangya Hospital of Cental South University

Hangzhou Xixi Hospital Affiliated to Zhejiang Chinese Medical University

Xixi Hospital of Hangzhou

Tongji Hospital, Tongji Medical College, Huazhong University of Science and Technology

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Zhongnan Hospital of Wuhan University

The First Affiliated Hospital, Zhejiang University School of Medicine

The First Affiliated Hospital, Zhejiang University School of Medicine

Hospital of Chengdu University of Traditional Chinese Medicine

Renmin Hospital of Wuhan University

Xiangyang 1st People's Hospital

Department and Institute of Infectious Disease

Zhongnan Hospital of Wuhan University

Xiangyang First People's Hospital

The First Affiliated Hospital of Medical College of Zhejiang University

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Tongji Hospital, Tongji Medical College, Huazhong University of Science and Technology

Hubei Integrated Traditional Chinese and Western Medicine Hospital
Huanggang Central Hospital (Dabieshan Medical Center)
People's Hospital of Guangshan County
Hanchuan People's Hospital
The First People's Hospital of Jiangxia District

Hubei Provincial Hospital of TCM

Hubei Provincial Hospital of TCM

The First Affiliated Hospital, Zhejiang University School of Medicine
The First Hospital of Jiaxing
Huzhou Central Hospital
The First People's Hospital of Xiaoshan District, Hangzhou
Wenzhou Central Hospital
Infectious Disease Hospital of Heilongjiang Province
Tongji Hospital
Harbin infectious diseases hospital
Anhui Province HOspital
The second hospital of Nanjing

Shanghai Public Health Clinical Center

The First Affiliated Hospital, College of Medicine, Zhejiang University

West China Hospital, Sichuan University
Chengdu Public Health Clinical Medical Center
Wuhan Red Cross Hospital
The Affiliated Hospital of Southwest Medical University
Guangqu District, the Maternal and Child Health Hospital of Hubei Province

Wuhan Pulmonary Hosptial

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The Fourth People's Hospital of Wuhan

TongJi Medical College of HUST

The Fifth Affiliated Hospital Sun Yat-Sen University

Tongji Hospital,Tongji Medical College, Huazhong University of Science and Technology
Institutional

The First Affiliated Hospital of Anhui University of Traditional Chinese Medicine
The First Affiliated Hospital Anhui Medicine University
Sixian Hospital of Traditional Medicine
Anhui Provincial Hospital

Tongji Hospital

Renmin Hospital of Wuhan University

Union Hospital, Tongji Medical College, Huazhong University of Science and Technology
Red Cross Hospital in Wuhan of Hubei Province
Wuhan Blood Center
Tongji Hospital, Tongji Medical College, Huazhong university of Science and Technology
Wuhan Asia Heart Hospital

The Second Affiliated Hospital of ChongQing Medical University
Three Gorges Central Hospital

Chongqing Public Health Medical Center
Three Gorge Central Hospital

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Chongqing Public Health Medical Center
Yongchuan Hospital Affiliated to Chongqing Medical University
Chongqing Shanxia Central Hospital
People's Hospital of Qianjiang District

Chongqing Public Health Medical Center
Yongchuan Hospital Affiliated to Chongqing Medical University
Chongqing Shanxia Central Hospital
People's Hospital of Qianjiang District

Wuhan Keting Square Cabin Hospital

The First Affiliated Hospital of University of science and technology of China (Anhui Provincial Hospital)

Zhongnan Hospital of Wuhan University

Enze Hospital of Taizhou Enze Medical Center (Group)

The First Affiliated of Wenzhou Medical University
The Second Affiliated of Wenzhou Medical University

Huangshi Hospital of traditional Chinese Medicine
Tongji Hospital of Huazhong University of science and technology
LaoHeKou Tradiational Chinese Medicine Hospital
Huanggang Central Hospital
Xishui County People's Hospital
Huangmei County People's Hospital
Wuhan Puren Hospital
Qichun County People's Hospital

Wuhan Puren Hospital
Huanggang Central Hospital
Xishui County People's Hospital
Huangmei County People's Hospital
Xiaogan Central Hospital

For peer review only

The First Clinical Hospital of Beijing University of Chinese Medicine (Dongzhimen Hospital)

The First Affiliated Hospital of He'nan University of Chinese Medicine
Zhengzhou Sixth People's Hospital
Nanyang Central Hospital
The First Affiliated Hospital of Nanyang Medical College
Zhumadian Central Hospital

Shanghai Pulmonary Hospital

The First Affiliated Hospital of Xinxiang Medical University

Renmin Hospital of Wuhan University

Wuhan Jinyintan Hospital (Wuhan Infectious Diseases Hospital)

Shanghai Public Health Clinical Center

Nanjing Second Hospital

Jingzhou Centeral Hospital

Union Hospital, Tongji Medical College, Huazhong University of Science and Technology

For peer review only

The Second Affiliated Hospital of Wenzhou Medical University

Jingzhou Centeral Hospital

The First Affiliated Hospital of Zhengzhou University

Wuhan Jinyintan Hospital
Renmin's Hospital of Wuhan University
Tongji Hospital Affiliated to Huazhong University of science and technology

People's Hospital of Guangshan County

The First Affiliated Hospital of Medical College of Zhejiang University

Beijing You'an Hospital, Capital Medical University
Huangshi Central Hospital of Hubei
Fuyang Second People's Hospital, Anhui
The Second Affiliated Hospital of Harbin Medical University
Heilongjiang Provincial Hospital
Suizhou Central Hospital
Xiaogan Central Hospital
Wuhan 7th Hospital
Ezhou Central Hospital
Jingzhou First People's Hospital
Huangshi Hospital of Traditional Chinese Medicine
Tongji Hospital of Huazhong University of science and technology
LaoHeKou Tradiational Chinese Medicine Hospital
Hubei province hospital of integrated Chinese & Western Medicine
Yichang first people's Hospital
Beijing YouAn Hospital
Renmin Hospital of Wuhan University
Tianyou Hospital Affiliated to Wuhan University of science and technology

For peer review only

Shuguang Hospital

Tongji Hospital of Tongji Medical College, Huazhong University of Science and Technology
The First Affiliated Hospital of Nanchang University
Beijing Haidian Hospital (Haidian Section of Peking University Third Hospital)

Tongji Hospital of Tongji Medical College, Huazhong University of Science and Technology
The First Affiliated Hospital of Nanchang University
Beijing Haidian Hospital (Haidian Section of Peking University Third Hospital)

HwaMei Hospital, University of Chinese Academy of Sciences

Wuhan Pulmonary Hospital
Wuhan Third People's Hospital
Hefei Infectious Disease Hospital
Jiangsu Province Infectious Disease Hospital
The First Affiliated Hospital of Xi'an Jiaotong University

Wuhan Pulmonary Hospital
Wuhan Third People's Hospital
Hefei Infectious Disease Hospital
Jiangsu Province Infectious Disease Hospital
The First Affiliated Hospital of Xi'an Jiaotong University

Union Hospital affiliated to Tongji Medical College of Huazhong University of Science and Technology
Hubei Hospital of traditional Chinese Medicine
Suizhou Central Hospital
The third people's Hospital of Hubei Province

Hospital of Chengdu University of Traditional Chinese Medicine

Tongji Hospital, Tongji Medical College, Huazhong University of Science and Technology

Shandong Province Chest Hospital
Jinan Infectious Disease Hospital
Jining Infectious Disease Hospital
Qingdao Chest Hospita
Yantai Qishan hospital

For peer review only

Yueyang Hospital of Integrated Traditional Chinese and Western Medicine, Shanghai University of Traditional Chinese Medicine
Huangshi Hospital of Traditional Chinese Medicine

Zhongnan Hospital of Wuhan University

Central theater General Hospital Wuchang district
Central theater General Hospital Hankou district
Huoshenshan hospital

beijing youan houspital
Wuhan PuRen Hospital
Shanghai Guanghua integrated traditional Chinese and Western Medicine Hospital

The First Affiliated Hospital of Nanchang Uninversity
Foshan 4th People's Hospital
Ezhou City Central Hospital

Guanggu District, Tongji Hospital, Tongji Medical College, Huazhong University of Science and Technology

The First Affiliated Hospital of Guangzhou Medical University
Xianning Central Hospital
First People's Hospital of Yunnan Province
Wuhan Sixth Hospital

Huangshi city hospital of TCM

The first affiliated hospital of Harbin medical university
The second affiliated hospital of Harbin medical university
The fourth affiliated hospital of Harbin medical university
Heilongjiang Provincial hospital

The First Affiliated Hospital of University of science and technology of China (Anhui Provincial Hospital)

For peer review only

Hospital of Chengdu University of Traditional Chinese Medicine

Union Hospital, Tongji Medical College, Huazhong University of Science and Technology

The First Affiliated Hospital of Guangzhou Medical University
Guangzhou Eighth People's Hospital
Union Hospital, Tongji Medical College, Huazhong University of Science and Technology
Wuhan Hankou Hospital

Wuhan Jinyintan Hospital (Wuhan Infectious Diseases Hospital)

The fourth people's hospital of nanning, guangxi

Jingzhou Hospital of Traditional Chinese Medicine
Wuhan Leishenshan (Thunder God Mountain) Hospital/The First Affiliated Hospital of Dalian Medical University
Xiangyang Hospital of Traditional Chinese Medicine

Beijing Chaoyang Hospital, Capital Medical University

Wuhan Fourth Hospital

First Affiliated Hospital of Anhui Medical University
Wuhan mobile cabin hospital
Binhu hospital of the first people's hospital of hefei
The second people's hospital of Hefei
Suzhou hospital affiliated to anhui medical university

Renmin Hospital of Wuhan University

For peer review only

The Sixth Affiliated Hospital of Guangzhou Medical University (Qingyuan People's Hospital)
Wuhan Dongxihu Fangcang Hosptial

The First Affiliated Hospital of Guangzhou Medical University

The first affiliated hospital of wenzhou medical university
Wenzhou central hospital
The first people's hospital of Wenling
Shaoxing people's hospital
Lishui zhongyi yiyuan
The first Affiliated Hospital of AnHui Medical University
Anhui Provincial Hospital for Infection
The First Affiliated Hospital of Bengbu Medical College
Henan Provincial People's Hospital
The First Affiliated Hospital of Zhengzhou University

GCP Office of The First Affiliated Hospital of Guangzhou Medical University

Guanggu District, Tongji Hospital, Tongji Medical College, Huazhong University of Science and Technology

The First Affiliated Hospital of Guangzhou Medical University
Wuhan University People's Hospital
Union Hospital Affiliated to Tongji Medical College of Huazhong University of science and technology
Wuhan Huangpi District Hospital of traditional Chinese Medicine
Renmin Hospital of Wuhan University
Zhongnan Hospital of Wuhan University
The Third Hospital of Wuhan City

Cancer Center, Xiehe Hospital,Tongji Medical College, Huazhong University of Science and Technology

People's Hospital of Wuhan University
Central Hospital of Chongqing Three Gorges
The First Affiliated Hospital of Chongqing Medical University
The Fourth People's Hospital of Taiyuan

For peer review only

The sixth medical center of PLA general hospital
The 5th medical center of PLA general hospital

Shanghai Changzheng Hospital

Tianjin Haihe Hospital
Zhongnan Hospital
Enshi Central Hospital

the first affiliated hospital of Nanchang university

Shanghai Public Health Clinical Center

Wuhan Longyang Hospital

Tongji Hospital, Tongji Medical College, Huazhong University of Science and Technology
The Second Affiliated Hospital of Xi'an Jiaotong University

Hainan Medical University

The 5th Medical Center Chinese PLA General Hospital

Renmin Hospital of Wuhan University

For peer review only

Nanhua Hospital Affiliated to Nanhua University

The First Affiliated Hospital of Nanchang University

Jingzhou first people's Hospital

Hubei Provincial Hospital of Integrated Traditional Chinese and Western Medicine

General Hospital of Central Theater Command
Maternal and Child Hospital of Hubei Province
Wuhan Huoshenshan Hospital

Puren Hospital Affiliated to Wuhan University of Science and Technology

West China Hospital of Sichuan University

The First Affiliated Hospital of Nanchang University, Ten layers pulse Research Institute of traditional Chinese medicine, Nanchang University

Fifth People's Hospital of Ganzhou

The First People’s Hospital of Jiangxia District
Union Hospital, Tongji Medical College, Huazhong University of Science and Technology

For peer review only

Wuhan Leishenshan Hospital

Zhongnan Hospital of Wuhan University
Leishenshan Hospital of Wuhan
The Third People's Hospital of Hubei Province

Leishenshan Hospital

Shanghai Changzheng Hospital

Ganzi Hospital of West China Hospital, Sichuan University

Wuxi Fifth People's Hospital

the First Affiliated Hospital of Harbin Medical University

Wuhan Jinyintan Hospital

Jingzhou Central Hospital

West China Hospital, Sichuan University

For peer review only

Union Hospital, Tongji Medical College, Huazhong University of Science & Technology

The second affiliated hospital of xi 'an medical university
Shaanxi provincial hospital for infectious diseases

Ruijin Hospital Affiliated to Shanghai Jiao Tong University School of Medicine

First people's hospital of Jiangxi district, Wuhan

Jingzhou First People's Hospital

Wuhan Jinyintan Hospital (Wuhan Infectious Diseases Hospital)

Harbin infectious diseases hospital

The First Affiliated Hospital of Fujian Medical University

Wuhan Raytheon mountain hospital (Zhongnan hospital, wuhan university)

The First Affiliated Hospital of Fujian Medical University

For peer review only

The Fourth People's Hospital of Wuhan

Wuhan Raytheon mountain hospital (Zhongnan hospital, wuhan university)

Leishenshan hospital

Maoming People's Hospital
Guangdong Provincial People's Hospital
Wuhan City Hankou Hospital
Jingzhou Central Hospital
Huizhou Central Hospital

Leishenshan Hospital
mobile cabin hospital

Affiliated Hospital of traditional Chinese and Western Medicine, Nanjing University of Chinese Medicine
Wuhan Jiangxia District First People's Hospital
Wuhan Jiangxia District Traditional Chinese Medicine Hospital
Affiliated Hospital of traditional Chinese and Western Medicine Nanjing University of Chinese Medicine
Wuhan Jiangxia District First People's Hospital
Wuhan Jiangxia District Traditional Chinese Medicine Hospital

Tongji Hospital

Zhongnan Hospital of Wuhan University
Wuhan Central Hospital
Wuhan Seventh Hospital

Hubei provincial hospital of integrated traditional Chinese and western medicine
Wuhan Hospital of Traditional Chinese Medicine

For peer review only

Hubei Shiyan Taihe hospital

Tetrandrine Tablets

The First Hospital of Harbin

Ethics Committee of the Second Affiliated Hospital of Wenzhou Medical University

Wuhan Red Cross Society Hospital
Hospital of Chengdu University of Traditional Chinese Medicine
Chengdu Fifth People's Hospital

Wuhan Red Cross Hospital

Tongji Hospital, Tongji Medical College, Huazhong University of Science and Technology

Hubei Hospital of Traditional Chinese Medicine
Xiaogan first people's Hospital
Xianning first people's Hospital
Xiehe Jiangnan hospital
The third people's Hospital of Hubei Province

Shandong Provincial Hospital

Wuhan Leishenshan Hospital

For peer review only

The First Affiliated Hospital of Zhengzhou University

Guiqiang Wang
Peking University First Hospital
Ezhou Central Hospital
Huoshenshan Hospital of Wuhan
Jinyintan Hospital of Wuhan

Liaocheng people's hospital

Qilu Hospital of Shandong University

Union Hospital affiliated to Tongji Medical College of Huazhong University of Science and Technology

The Third People's Hospital of Shenzhen

Union Hospital, Tongji Medical College, Huazhong university of Science and Technology
Red Cross Hospital in Wuhan of Hubei Province
Wuhan Asia Heart Hospital
Wuhan Maternal and Child Health Hospital

Xiangya Hospital (West District of Wuhan Union Medical College Hospital)
Third Xiangya Hospital (Sino-French New Hospital District of Wuhan Tongji Hospital)

The first people's Hospital of Jiangxia District, Wuhan

Zhongnan Hospital of Wuhan University

For peer review only

Hubei Provincial Hospital of TCM

Shenzhen Third People's Hospital (infection department 2)
Shenzhen People's Hospital (infection department)

The First Affiliated Hospital of Wenzhou Medical University
Wenzhou city center hospital medical group
The second hospital affiliated to wenzhou medical college

Shanghai Changzheng Hospital
Guanggu District, Hubei maternal and Child Health Hospital

Maoming People's Hosptial

Wuhan No.7 Hospital/Jiznong Energy Fengfeng Group Hospital
Wuhan No.7 Hospital/North China University of Science and Technology Affiliated Hospital
Zhongnan Hospital of Wuhan University/Tanshan People's Hospital
Wuhan Leishenshan (Thunder God Mountain) Hospital/The First Affiliated Hospital of Dalian Medical University

The First Affiliated Hospital of Guangzhou Medical University

HwaMei Hospital, University of Chinese Academy of Sciences

HwaMei Hospital, University of Chinese Academy of Sciences

Foshan First People's Hospital
Zhongshan Second People's Hospital
The Ninth People's Hospital of Dongguan
Jiangmen Central Hospital
Foshan Fourth People's Hospital

For peer review only

Affiliated Hospital of Shaanxi University of Traditional Chinese Medicine

Renmin Hospital of Wuhan University

The First Affiliated Hospital of Nanchang University
Xinyu People's Hospital
Jiujiang No.1 People's Hospital
The First Affiliated Hospital of Guangzhou Medical University
Zhengzhou Sixth People's Hospital
Public Health Clinical Center of Chengdu
The First Hospital of Qiqihar
Heze Municipal Hospital

The Second Affiliated Hospital of Nanchang University
Jiangxi Chest Hospital

Beijing Chaoyang hospital

Hospital of Leishenshan

Wuhan Jinyintan Hospital

Wuhan Tongji Hospital Optical Valley Branch

The Third Xiangya Hospital of Central South University
The Fourth People's Hospital of Yueyang

For peer review only

Shanghai Public Health Clinical Center

Guangzhou Institute of Respiratory Health, The First Affiliated Hospital of Guangzhou Medical University

Beijing You'an Hospital, Capital Medical University
Peking University First Hospital
Beijing huimin hospital

First Affiliated Hospital of Guangzhou Medical University

Chongqing Public Health Medical Center

Fuzhou General Hospital

The Third Affiliated Hospital of Sun Yat-sen University

Wuhan Jiangxia Cabin Hospital

Renmin Hospital of Wuhan University (East Campus)

Tongji Hospital of Tongji Medical College Huazhong Science and Technology University

For peer review only

The fifth affiliated hospital of sun yat-sen university

The Second People's Hospital of Fuyang
Ezhou Hospital of Traditional Chinese Medicine
Ezhou Central Hospital
Huoshenshan Hospital of Wuhan
Jinyintan Hospital of Wuhan
The Second People's Hospital of Fuyang
Ezhou Central Hospital
Huoshenshan Hospital of Wuhan
Jinyintan Hospital of Wuhan
Tongji Hospital of Huazhong University of Science and Technology

Leishenshan hospital

Huangzhou District People's Hospital of Huanggang City

Hubei Provincial Center for Disease Control and Prevention

Huzhou central hospital

Wuzhi County Center for Disease Control and Prevention
People’s hospital, Wuzhi county

Jiangsu Province Hospital
Hubei Province Hospital Intergrated Chinese & Western Medicine
Huangshi Hospital of Chinese Traditional Medcine
Guangzhou Eighth People's Hospital
Maoming People's Hospital
Shenzhen Third Peoples Hospital
Yuebei Second People's Hospital
Dongguan Ninth People's Hospital

For peer review only

Suining County Center for Disease Control and Prevention

China Academy of Chinese Medical Sciences?

Huangzhou District People's Hospital of Huanggang

Huoshenshan Hospital

Shanghai Public Health Clinical Center

Hubei Province Hospital

Hubei Provincial Hospital of TCM
Wuchang District Shuiguohu street community health service center
Wuchang District Xujiapeng street community health service center

Wuhan Hospital Of Traditional Chinese and Western Medicine
Wuhan No. 7 hospital
Xiaogan Traditional Chinese Medicine Hospital

Hubei Provincial Hospital of TCM
Wuhan Third Hospital
Xiaogan Chinese Medical Hospital

The First Affiliated Hospital of Harbin Medical University
Hongqi Hospital Affiliated to Mudanjiang Medical College

For peer review only

Cai Yue

Beijing University of Chinese Medicine Third Affiliated Hospital
China-Japan Friendship Hospital
Berjing Dong zhi men Hospital
West China Hospital, Sichuan University
Changhai hospital
Wuhan Red Cross hospital
East hospital of people's hospital of Wuhan University
Wuhan Wuchang hospital
Yueyang Integrated Traditional Chinese and Western Medicine Hospital Affiliated to Shanghai University of Traditional Chinese Medicine
Huangshi City Hospital of Traditional Chinese Medicine
Xiaogan Traditional Chinese Medical Hospital
Hubei Provincial Hospital of TCM
Ezhou hospital of traditional Chinese medicine
West Campus of Union Hospital affiliated to Tongji Medical College of Huazhong University of Science and Technology
Liangyuan district centers for disease control and prevention
Wuhan Jinyintan hospital
The First Affiliated Hospital of Guangzhou Medical University
Xiangzhou District People's Hospital
Third Hospital of Baotou City
Wuhan Pulmonary Hospital
Xiangyang Central Hopital
Hubei Provincial Hospital of TCM

For peer review only

The First Affiliated Hospital of Guangdong Pharmaceutical University
Guangzhou Eighth People's Hospital

Shanghai Public Health Clinical Center
Guangzhou 8th People's Hospital
TIANJIN HAIHE HOSPITAL
The First Affiliated Hospital of Harbin Medical University
Harbin Sixth Hospital

Renqiu City Center for Disease Control and Prevention

Nanjing Drum Tower Hospital

Hongqi Hospital Affiliated to Mudangjiang Medical University

Wuhan Third Hospital
Wuhan Integrated Traditional Chinese and Western Medicine Hospital (Wuhan First Hospital)
Ezhou Hospital of Traditional Chinese Medicine

First Hospital of China Medical University
China-Japan Friendship Hospital
Xiangya Second Hospital, Central South University

The First Affiliated Hospital of Wenzhou Medical University

West China Second University Hospital, Sichuan University
West China women's and children's Hospital

Wuhan Sports Center makeshift hospital
Jiangsu Province Hospital

For peer review only

Guangzhou Women and Children’s Medical Center
Guangzhou Eighth People's Hospital

Xiangyang Central Hospital
First People's Hospital of Xingyang

Yueyang Integrated Traditional Chinese and Western Medicine Hospital Affiliated to Shanghai University of Traditional Chinese Medicine
Huangshi City Hospital of Traditional Chinese Medicine

Shuguang Hospital Affiliated to Shanghai University of Traditional Chinese Medicine

Huashan Hospital affiliated to Fudan University

Children's Hospital Affiliated to Zhejiang University School of Medicine

The Second Affiliated Hospital of Chongqing Medical University

The First Affiliated Hospital of Bengbu Medical College

Shulan (Hangzhou) Hospital
Yongfu County Center for Disease Control and Prevention

Hunan Provincial Center for Disease Control and Prevention

For peer review only

West China Second University Hospital, Sichuan University
West China women's and children's Hospital

Investigative Site

Investigative Site

Beijing SHIJITAN Hospital

Hubei Hospital of Traditional Chinese Medicine

Shanghai Public Health Clinical Center

Hunan Provincial Center for Disease Control and Prevention

Jiangsu Provincial Center for Disease Control and Prevention

Shanghai Public Health Clinical Center

Jiangsu Provincial Center for Diseases Control and Prevention

For peer review only

Wuhan Jinyintan Hospital

Dongtai Center for Disease Control and Prevention

Huashan Hospital Affiliated to Fudan University

Zanhuang county Center for Disease Control and Prevention

Hubei 672 Orthopaedic Hospital of Intergrated Traditional Chinese and Western Medicine
The Third Affiliated Hospital of Beijing University of traditional Chinese Medicine

Hubei 672 Orthopaedic Hospital of Integrated Traditional Chinese and Western Medicine
Beijing University of Chinese Medicine Third Affiliated Hospital

Taixing City center for Disease Control and Prevention

Beijing Ditan Hospital affiliated to Capital Medical University
Huashan Hospital affiliated to Fudan University

Hubei 672 Orthopaedics Hospital of Integrated Chinese and Western Medicine

Huaiyin District Center for Disease Control and Prevention, Huaian City

For peer review only

Guangzhou Institute of Respiratory Disease
First Affiliated Hospital of Guangzhou Medical University

Shulan (Hangzhou) Hospital

Xiangyang Central Hospital
Xiangyang Hospital of Traditional Chinese Medicine

Huaiyin District Center for Disease Control and Prevention, Huaian City

Dafeng District Center for Disease Prevention and Control

Jiangsu Dongtai Center for Disease Control and Prevention

Shanghai Public Health Clinical Center

Jiangsu Provincial Center for Disease Control and Prevention

Jiangsu Provincial Center for Diseases Control and Prevention

Jiangsu Provincial Center for Diseases Control and Prevention

For peer review only

Danyang City Center for Disease Control and Prevention

Xiangtan County Center for Disease Control and Prevention

Xi'an International Medical Center Hospital

Shanghai Public Health Clinical Center

Investigative Site

Taiyuan Center for Disease Control and Prevention
Shanxi Bethune Hospital
Taiyuan People's Hospital
Taiyuan Fourth People's Hospital

Outpatient Department of Shanxi Aviation Industry Group Co. LTD

Outpatient Department of Shanxi Aviation Industry Group Co. LTD

Yongfu CDC
Xiangfen CDC

The Second Hospital of Xingtai
Harbin Infectious Disease Hospital

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The Second Hospital of Xingtai
Harbin Infectious Disease Hospital

The Second Hospital of Xingtai
Harbin Infectious Disease Hospital

Shu Lan (Hang Zhou) Hospital

Jiangsu Provincial Center for Diseases Control and Prevention

Shijiazhuang Fifth Hospital
Shijiazhuang People's Hospital

The Second Hospital of Hebei Medical University

Hubei Provincial Hospital of Traditional Chinese Medicine

Beijing Ditan Hospital Capital Medical University
Guangzhou Eighth Hospital, Guangzhou Medical University
The First Affiliated Hospital of Guangzhou Medical University
The Third People's Hospital of Shenzhen
Subei People's Hospital of Jiangsu province

Shanghai Oriental Hospital

Kaihua county Center for Disease Control and Prevention

For peer review only

Jiangsu Provincial Center for Diseases Control and Prevention

Public Health Clinical Center of Chengdu (Chengdu Infectious Disease Hospital)

Jiangsu Provincial Center for Diseases Control and Prevention

Gaozhou Municipal Center for Disease Control and Prevention

Gaozhou Municipal Center for Disease Control and Prevention

The First Affiliated Hospital, Sun Yat-Sen University

Jiangsu Provincal Center for Disease Control and Prevention

Ning ling Center for Disease Control and Prevention

Yanjin County Center for Disease Control and Prevention

shulan (Hangzhou) Hospital

For peer review only

Zanhuang county Center for Disease Control and Prevention

Yuepu Community Health Service Center
Kangjian Community Health Service Center
Xinzhuang Community Health Service Center
Dongjing Town Community Health Service Center
Sijing Town Community Health Service Center

Xiangzhou District Center for Disease Control and Prevention
Liangyuan District Center for Disease Control and Prevention
Yangchun Center for Disease Control and Prevention

Shijiazhuang People's Hospital
The Fifth Hospital of Shijiazhuang City
Xingtai Second People's Hospital

General Hospital of Northern Theater Command

Funing Center for Disease Control and Prevention

Jiangsu Provincial Center for Disease Control and Prevention

Jiangsu Dongtai Center for Disease Control and Prevention

Shanghai East Hospital

the Eighth Affiliated Hospital of Guangzhou Medical University

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Shanxi Center for Disease Control and prevention
Sichuan Center for disease control

The Second Hospital of Anhui Medical University

Rushan City Center for Disease Control and Prevention

Hunan Provincial Center for Disease Control and Prevention

Xiangcheng Center for Disease Control and Prevention

Xiangcheng Center for Disease Control and Prevention

Huaiyin Center for Disease Control and Prevention
Binhai Center for Disease Control and Prevention
Xiangshui Center for Disease Control and Prevention

Hunan Provincial Center for Diseases Control and Prevention

Hunan Provincial Center for Diseases Control and Prevention

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Jiangsu Provincial Center for Diseases Control and Prevention

Xiangtan County Center for Disease Control and Prevention

Wuhan Jiangxia District Hospital of Traditional Chinese Medicine
Wuhan First Hospital
Huai 'an Traditional Chinese Medicine Hospital
Zhenjiang Infectious Disease Hospital
Yangzhou Hospital of Traditional Chinese Medicine

The Second Hospital of Nanjing
Yangzhou Third People's Hospital

Jiangsu Provincial Hospital of Chinese Medicine
The second hospital of Nanjing

Jiangsu Privince Hospital of Chinese Medicine(centralized isolation site under its management)
Yangzhou Hospital of Traditional Chinese Medicine (centralized isolation site under its management)

Center for Disease Control and Prevention of Xiangyun County

Yuncheng Yanhu District Center for Disease Control and Prevention

The First Affiliated Hospital of Chongqing Medical University

Shulan(Hangzhou)Hospital

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Yongfu County Center for Disease Control and Prevention
Liujiang District Center for Disease Control and Prevention
Yun County Center for Disease Control and Prevention

Hubei Provincial Hospital of Integrated Chinese and Western Medicine

Ruijin Hospital, Medical School of Shanghai Jiaotong University

Shanghai Public Health Clinical Center

Huangshi Hospital of TCM

Donghai County Center for Diseases Control and Prevention

Hubei Provincial Hospital of Integrated Chinese and Western Medicine

Jiangsu Provincial Center for Diseases Control and Prevention

Beijing Centers for Diseases Control and Prevention

Guangdong Center for Disease Control and Prevention
Henan Center for Disease Control and Prevention
Hubei Center for Disease Control and Prevention

For peer review only

Shi Jiazhuang People's Hospital
The First Affliated Hospital of Harbin Medical University
Suihua first hospital

Xinqi City Center for Disease Control and Prevention

Hunan Provincial Center for Disease Control and Prevention

Taihe Hospital of Shiyan City

Wuhan Jinyintan Hospital

Xiangtan County Center for Disease Control and Prevention

Beijing Ditan Hospital, Capital Medical University

The First Affiliated Hospital of Zhengzhou University

Shanghai Public Health Clinical Center

Hunan Provincial Center for Disease Control and Prevention

For peer review only

Liuzhou County Center for Disease Control and Prevention

Yangchun City Center for Disease Control and Prevention
Zhanjiang Center for Diseases Control and Prevention

Neijiang City Center for Disease Control and Prevention

Shanghai Public Health Clinical Center

Yaliang county Center for Disease Control and Prevention

Huai'an No.4 People's Hospital
Taizhou People's Hospital
Wuxi No.5 People's Hospital
Xuzhou Infectious Disease Hospital
Binhai County Hospital of Chinese medicine

Yanhe Tujia Autonomous County Center for Disease Control and Prevention

Beijing Youan Hospital Capital Medical University

The Second Affiliated Hospital of Guangzhou Medical University

Center for Disease Control and Prevention of Xiangyun County

For peer review only

Huashan Hospital affiliated to Fudan University

Beijing Pinggu Hospital
Peking University Health Science Center Hospital

Beijing Pinggu Hospital
Peking University Health Science Center Hospital

Hubei Provincial Hospital of Traditional Chinese Medicine
Wuhan NO.7 Hospital
Wuhan NO.1 Hospital

Panzhihua Municipal Center for Disease Control and Prevention

Beijing Ditan Hospital, Capital Medical University

Lancang County Center for Disease Control and Prevention

Affiliated Hospital of Yangzhou University

Danyang City Center for Disease Control and Prevention

Jiangsu Provincial Center for Diseases Control and Prevention

For peer review only

Shanghai Xuhui Central Hospital

Huashan Hospital affiliated to Fudan University
Beijing Ditan Hospital Capital Medical University

Anhui Provincial Hospital
Peking University First Hospital
Beijing Tongren Hospital, CMU
PetroChina Central Hospital
Hunan Provincial Center for Disease Control And Prevention

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Chang'an District Hospital of The First Affiliated Hospital of Xi'an Jiaotong University

The first people's hospital of changde city

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Province Involved in Research	Randomization	Masking
Hubei	Prior to enrollment, the researchers rechecked the selection and exclusion criteria to determine if the subjects were	Open label
Beijing	#NOTREQ	None (Open Label)
Beijing	#NOTREQ	None (Open Label)
Chongqing	Subjects included in the study after clinical diagnosis by the research leader, then the third-party statisticians use Excel to generate	Not stated
Chongqing	Subjects included in the study after clinical diagnosis by the research leader, then use Excel to generate random	Not stated
Shanghai	#NOTREQ	None (Open Label)
Hubei	Entrusting a third-party company, using a simple randomization method to generate random sequences	Not stated
Jiangsu	#NOTREQ	Quadruple (Participant, Care Provider, Investigator, Outcomes Assessor)
Guangdong	#NOTREQ	None (Open Label)

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Hubei	#NOTREQ	Quadruple (Participant, Care Provider, Investigator, Outcomes Assessor)
Beijing	#NOTREQ	Quadruple (Participant, Care Provider, Investigator, Outcomes Assessor)
Liaoning Hubei	Parallel random	Not stated
Hubei Hebei Beijing Henan Chongqing	Statistical professionals use a computer to generate a table of random numbers, and all patients are numbered by the doctor in	Not stated
Hubei Anhui Guizhou Heilongjiang Zhejiang	Randomization numbers were generated using SAS statistical software package (SAS Inc., Cary, USA). A computer generated 1:1	Open label
Hubei	The random number table method was adopted by the third party for random grouping	Not stated
Hubei	Evidence based medicine center at Tianjin University of traditional Chinese medicine will generates the random number using SAS software	Not stated
Hubei	Evidence based medicine center at Tianjin University of traditional Chinese medicine will generates the random number using SAS software	#BLANK
Hubei	SPSS was used to generate random sequences	Not stated
Hubei	SPSS was used to generate random sequences	Open label

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Hubei	SPSS was used to generate random sequences	Not stated
Hubei	SPSS was used to generate random sequences	Not stated
Hubei	SPSS was used to generate random sequences	Open label
Hubei	#NOTREQ	Single (Participant)
Hubei	#NOTREQ	Single (Participant)
Hubei	#NOTREQ	Single (Participant)
Hunan	The statistician uses SAS software to generate the random assignment table, and the researcher obtains the random number	Not stated
Zhejiang	Use stratified random method. Taking the underlying disease of organ damage as a stratification factor, a set of random	Double blind.
Zhejiang	A stratified block random method was used. Based on the 2019-nCoV pneumonia general type (early TCM clinical treatment) and heavy	Double blind
Hubei	Prior to enrollment, the researchers rechecked the selection and exclusion criteria to determine if the subjects were	open label

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Hubei	After the subjects are enrolled, random numbers are generated by software according to the block randomization method	Not stated
Zhejiang	Prior to enrollment, the researchers rechecked the selection and exclusion criteria to determine if the subjects were enrolled. SAS	Not stated
Zhejiang	Prior to enrollment, the researchers rechecked the selection and exclusion criteria to determine if the subjects were enrolled. SAS	open label
Sichuan	The randomization was implemented using the MagMinDA central random grouping system of "Xi'an Combarav Life Science	Not stated
Hubei	By trained professionals, numbered and grouped by random indicator method, in order to the patient number (according to the inclusion	Double blind.
Hubei	The R software was used to generate random Numbers at 1:1, and the generated Numbers were placed in the envelope with opaque light	Open label
Hubei	#NOTREQ	Single (Participant)
Hubei	#NOTREQ	Triple (Participant, Care Provider, Outcomes Assessor)
Hubei	The R software was used to generate random Numbers a t 1:1, and the generated Nu mbers were placed in the en velope with opaque light and	Blind method for subjects.
Zhejiang	Prior to the study, a statistician wrote the randomization protocol and, according to the randomization protocol, used	open-label

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Hubei	Random number table	single blind
Hubei Henan Guangxi Jiangsu	Non-randomization.	open label
Hubei	Subjects will be?assigned to the treatment?group and the control group on a 1:1 basis? by simple randomization. Tw o groups of random?number	N/A
Hubei	Subjects are assigned to treatment group and control group on a 1:1 basis by simple randomization. Two groups of random numbers	Open label
Zhejiang	In this trial, subjects were assigned a random number and a corresponding study drug using a central random method	Open label
Heilongjiang Hubei Anhui Jiangsu	Statistical professionals use a computer to generate a table of random numbers, and all patients are numbered by the doctor in	open label
Shanghai	#NOTREQ	None (Open Label)
Zhejiang	In this trial, subjects were assigned a random number and a corresponding study drug using a central random method	open label
Sichuan	The drugs were randomly coded.Each hospital enrolled subject was randomly coded into either the experimental or control group	Single blind
Hubei	In this study, telepnone / WeChat randomization was used. In order to ensure the concealment of distribution, the personnel responsible for	open label

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Hubei	Computer-generated sequence of random numbers	Not stated
Hubei	By trained professionals, numbered and grouped by random indicator method, the patient id number was acquired by time who was	Not stated
Guangdong	Prior to the start of the experiment, a statistical expert randomly assigns 1 to 112 numbers according to the statistical software to	Open label
Hubei	Computer software SAS to be used to generate the random number sequence, stratification by the characteristics of the	Not stated
Anhui	Random sequence is generated by the test project team's chargehand according to simple random	This test is an open test, no blind method.
Hubei	This study use randomized, open and parallel design. Whether the subject will enter into the control group or test group in the study will	open label
Hubei	Random grouping: The proc plan process of SAS9.4 software was used to generate random numbers and randomly divided into	Because Xiyanping injection and α-interferon have slight differences in appearance and color; this study uses an open design, but during the research, it is recommended that researchers who have nothing to do with the
Hubei Shanghai	The randomization method of this study is as follows: Subjects are randomly assigned to the experimental group (conventional	There is no blind method in this study.
Chongqing	SPSS was used to generate random sequences.	open label
Chongqing	In this study, a central randomization system will be used to allocate the patients to the experimental or control group at a ratio of 1:1 after	open label

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Chongqing:	The random number table generated by the PROC PLAN command in the SAS software, the automatically generated group, and the	open label
Chongqing:	The random number table generated by the PROC PLAN command in the SAS software, the automatically generated group, and the	Not stated
Hubei	Random number table was used to generate random sequences by statisticians from China Center for Evidence-Based Traditional	open label
Anhui,	The procpian program of SAS software was used to generate random numbers and randomly them divided into two groups, namely the	Not stated
Hubei	The statistician uses SAS software to generate the random assignment table, and the researcher obtains the random number	open label
Zhejiang	random number table	Not stated
Zhejiang	All patients with Novel coronavirus pneumonia were randomly divided into group A and group B by computers, and both the investigators	open label
Hubei	Random grouping method: the eligible subjects were selected from each center and consecutively grouped according to the proportion	Not stated
Hubei	The statisticians used a method of stratification and randomization by center.	open label
Hubei	The statisticians used a method of stratification and randomization by center	open label

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Beijing	Sixty patients who met the inclusion criteria were divided into a treatment group and a control group with 30 cases each	Not stated
Henan	Central random assignment method	Not stated
Shanghai	Generate 120random numbers with SPSS23.0 statistical analysis system.	Not stated
Henan	#NOTREQ	None (Open Label)
Hubei	The block random method is adopted, and the random number is generated by using the proc plan procedure statement of the	Because hydroxychloroquine and abidol hydrochloride are significantly different in appearance and color; this study uses an open design, but during the research, medicines are distributed by researchers who
Hubei	According to different research centers and whether to use stratified randomization with hormones and randomized	Not stated
Shanghai	Block randomization method will be used. The professional statistical software will be used to generate a random list of	Open label
Jiangsu	excel for randomization.	no-blinding
Hubei	The statistician uses SAS software to generate the random assignment table, and the investigator obtains the random number	Not stated
Hubei	#NOTREQ	None (Open Label)

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Zhejiang	#NOTREQ	None (Open Label)
Hubei	The statistician uses SAS software to generate the random assignment table, and the investigator obtains the random number	Not stated
Henan	Random number table	Not stated
Hubei	The procplan program of SAS software was used to generate random numbers and randomly them divided into two groups, namely the	Not stated
Henan	In this study, subjects were randomly assigned to the experimental group or control group.The randomization table was	Not stated
Zhejiang	Random number table method	Not stated
Beijing Hubei Anhui Heilongjiang Liaoning	using computer statistical software R to generate random numbers.	open label
Hubei Anhui Sichuan Henan	Statisticians generate random sequences and use randomization envelopes to randomize subjects.	Not stated
Hubei	Random grouping method: the eligible subjects were selected from each center and consecutively grouped according to the proportion	Not stated
Hubei Beijing	#NOTREQ	None (Open Label)

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Shanghai	#NOTREQ	Triple (Participant, Care Provider, Investigator)
Hubei Jiangxi Beijing	central randomization	open label
Hubei Jiangxi Beijing	central randomization	open label
Zhejiang	Professional statisticians generate random numbers by Statistical Product and Service Solutions (SPSS) 19.0	Not stated
Hubei Anhui Jiangsu Shaanxi	Some professor from Fudan University used the stratified method (taking center as stratified factor) and block random method. Using the	Not stated
Hubei Anhui Jiangsu Shaanxi	Some professor from Fudan University used the stratified method (taking center as stratified factor) and block random method. Using the	Not stated
Hubei	For patients eligible for randomization, each study center will be assigned random codes in strict order. The random code will be	Open label
Sichuan	Eligible subjects will receive a random number from small to large according to the screening number. Using the SAS9.4 software PLAN	Not stated
Hubei	Table of random numbers	Not stated
Shandong	According to the age group and clinical type of the subjects, the stratified area group of the subjects was randomly assigned to the	Open label

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Shanghai Hubei	Clinical researchers will get a random sequence number which is automatically generated by a random number generator	The trainers will not know the assessment of outcomes. To reduce the risk of bias, evaluators, data managers, and statisticians were unaware of group assignments in the outcome evaluation process and data
Hubei	Statistical experts use R software to generate random results and put them into numbered envelopes in sequence. After the	open label
Hubei	Complete random design by major researchers using computers	Not stated
Beijing	GUANGLIANG SHAN Completely random grouping	
Hubei Shanghai	Randomly divided into group A as placebo group and group B as stem cell	Not stated
Jiangxi Guangdong	This experiment adopts bloc k random method. Statistics personnel with the aid of stati	open label
Hubei	stical software SAS 9.4 PRO C PLAN process statements	
Hubei	The researchers randomly generated a set of random integers with a range of 2:2:1 in Excel table.	open label
Guangdong Hubei Yunnan	This trial use the Central Randomization System, the expert generate randomization by SAS sofeware	open-label
Hubei	Clinical researchers will get a random sequence number which is automatically generated by a random number generator	open label
Heilongjiang	Professor Li kang, Department of statistics, The first affiliated hospital of Harbin medical university, generates the random	Double blinding
Anhui	The procplan program of SAS software was used to generate random numbers and randomly them divided into two groups, namely the	Not stated

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Sichuan	cluster randomization	Not stated
Hubei	Table of random numbers	Blind method for subjects.
Guangdong Hubei	Patients were randomly coded by the sponsor, and were distributed to experimental or control groups randomly. The	Not stated
Hubei	The patients were stratified according to the respiratory support conditions and randomly assigned to the test group and the control	Not stated
Guangxi	The researchers used SPSS software to generate random numbers	Double blind
Hubei	#NOTREQ	Double (Participant, Outcomes Assessor)
Beijing	This trial will use a dynamic random approach by statisticians	open label
Hubei	Using random number table method	Open label
Anhui Hubei	Professional statisticians generate random numbers by Statistical Product and Service Solutions (SPSS) 17.0	Open label
Hubei	For patients eligible for randomization, study center will be assigned random codes in strict order. The random code will be	Not stated

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Guangdong Hubei	The statistician uses SAS software to generate the random assignment table, and the investigator obtains the random number	Double blind
Guangdong	An independent researcher used a computer-generated random number table to randomly assign eligible COVID-19 close contacts	Open-labelled
Zhejiang	Using SPSS to generate random sequences，completed by team member Yanli Wan.	Not stated
Anhui Henan Hubei Hunan Jiangxi	stratified blocked randomization	Not stated
Guangdong	#NOTREQ	None (Open Label)
Hubei	The researchers randomly generated a set of random integers with a range of 2:2:1 in Excel table.	Not stated
Guangdong Hubei Zhejiang	Clinicians in each center generate random sequences by simple randomized grouping.	Not stated
Hubei	Random number table method	Double blind method, both the doctors and the patients don't know the medicine information.
Hubei	By trained professionals, numbered and grouped by random indicator method, in order to the patient number (according to the inclusion	open label
Hubei Chongqing Shanxi	Resman (www.medresman.ORG), a public management platform for clinical trial management of China clinical trial registration	Not stated

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Beijing	The third party USES computer software to generate the random distribution table randomly, and the third party is	Not stated
Shanghai	The procplan program of SAS software was used to generate random numbers and randomly them divided into two groups, namely the	Not stated
Tianjin Hubei	Randomized parallel trial	Not stated
Jiangxi	Generated by a table of random numbers	Not stated
Shanghai	A stratified block randomization method was used. The stratified variables were age and whether antiviral drugs had been	Open label
Hubei	The RAND function in Excel was used to generate random sequences. The random sequences were put into opaque kraft envelopes	Not stated
Hubei Shaanxi	Randomized grouping，random sequence table.	Not stated
Hainan	In order to reduce possible selective bias and curative effect bias from researchers and subjects, this study will use a randomized, double-	Double blind
Beijing	Before subjects were enrolled, researchers used SPSS 24.0 software to generate random Numbers and serial Numbers, and	Open label, Non-blinding.
Hubei	Block randomization method will be used. The professional statistical software will be used to generate a random list of	Not stated

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Hunan	random number table	Not stated
Jiangxi	Random number table	Not stated
Hubei	Random number table generated by professional statistical software	Not stated
Hubei	EBM & Clinical Research Service Group of Guangdong Provincial Hospital of Chinese Medicine will generates the random	Not stated
Hubei	#NOTREQ	Quadruple (Participant, Care Provider, Investigator, Outcomes Assessor)
Hubei	#NOTREQ	Triple (Participant, Care Provider, Outcomes Assessor)
Sichuan	SPSS was used to generate random sequences.	Not stated
Jiangxi	Not stated	Not stated
Jiangxi	Random grouping using SAS9.4 software	#BLANK
Hubei	The project leader generates random sequences from a table of random digit.	Not stated

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Hubei	Random number is from random number table	Open label
Hubei	SAS9.4 generated the random number sequence.	Not stated
Hubei	parallel	Not stated
Shanghai	Parallel study	Not stated
Sichuan	The experimenters were divided into groups by random number table. After each patient was randomly arranged and numbered, the	Open label
Jiangsu	Random envelope method	Not stated
Heilongjiang	In this study, patients with high inflammation will be randomly divided into two groups according to the ratio of 1: 1. The length of the	Not stated
Hubei	SAS 9.4 software was used to generate random sequences by statisticians from China Center for Evidence-Based Traditional	open label
Hubei	According to the above included diagnosis of severe and critical patients, and the new coronavirus positive 200 patients, patients will be	Not stated
Sichuan	The research assistants who were not involved in case inclusion first used excel to generate 300 random Numbers (according to	Open label

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Hubei	A biostatistician who has no relationship with the statistical analysis of data management in this experiment, using a SAS	Not stated
Shaanxi	The researchers used a dice roll to randomly assign the enrolled patients.	Open label
Shanghai	#NOTREQ	Triple (Participant, Care Provider, Investigator)
Hubei	In this study, cases are randomly assigned to experimental and control groups at a ratio of 1: 1 by block randomization	Open label
Hubei	Random number table method	Not stated
Hubei	The IWRS random system is adopted, and the hierarchical block randomization is adopted, and the group is obtained in chronological	Double blind
Heilongjiang	The random number table used in this test plan was generated by biostatistics professionals using professional statistical	Not stated
Fujian	Random number table	Open label
Hubei	The third party statisticians used SAS software to generate random sequences using the method of stratified randomization	Open label
Fujian	80 clinical first-line nurses were selected. The subjects were divided into experimental group (40) and control group (40) by parallel	Open label

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Hubei	Computer-generated sequence of random numbers	Open label
Hubei	The third party statisticians used SAS software to generate random sequences using the method of stratified randomization	Open label
Hubei	A random number table is generated by the computer, and the subjects are numbered sequentially according to the order of the	#BLANK
Guangdong Hubei	Recruited patients were randomly assigned to each hospital by telephone verification in a ratio of 1:1. Four patients in each district	Single blind
Hubei	Stratified block randomization	Open label
Jiangsu Hubei	The patients who meet the inclusion criteria and do not meet the exclusion criteria will be sorted according to the order of admission, and	Open label
Jiangsu Hubei	The patients who meet the inclusion criteria and do not meet the exclusion criteria will be sorted according to the order of admission, and	open label
Hubei	Central randomization was used in this study. In order to ensure the concealment of distribution, personnel responsible for	Open label
Hubei	The researchers number the qualified subjects according to the order of their visits, and divide them into the corticosteroid therapy group	Not stated
Hubei	This study is a prospective randomized clinical trial using random stratification.Enrolled patients were randomly divided into two groups.The contr	#BLANK

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Hubei	In this experiment, the simple random grouping method was used.	#BLANK
Zhejiang	#NOTREQ	None (Open Label)
Heilongjiang	Statistical professionals use a computer to generate a table of random numbers, and all patients are numbered by the doctor in	open label
Zhejiang	Statistical professionals use a computer to generate a table of random numbers, and all patients are numbered by the doctor in	Not stated
Hubei Sichuan	SAS was used to generate random sequences. Each hospital enrolls subject will be randomly coded into either the experimental or	Double blind
Hubei	A multicenter, randomized, parallel controlled trial design was used in this study.The research in each research center is random The study	Blind method for subjects.
Hubei	The researchers number the qualified subjects according to the order of their visits, and divide them into the corticosteroid therapy group	Not stated
Hubei	For patients eligible for randomization, each study center will be assigned random codes in strict order. The random code will be	Not stated
Shandong	The clinicians of this study used the EXCEL table to generate random sequences before patients were enrolled, and were divided	Open label
Hubei	Computer random number	#BLANK

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Henan	Random number table	Not stated
Beijing Hubei	#NOTREQ	None (Open Label)
Shandong	Use Excel to generate random Numbers to generate random sequences	N/A
Shandong	#NOTREQ	Triple (Participant, Care Provider, Outcomes Assessor)
Hubei	This trial will use a group random approach by authorized staff.	open label
Guangdong	This trial will use a group random approach by authorized staff.	Open label
Hubei	The randomization method of this study is as follows: Subjects are randomly assigned to the experimental group (conventional	There is no blind method in this study.
Hunan	Centralized randomization, using the eLite stochastic system of Nanjing Medical University, randomly assigned patients to the	The primary outcome measures CT results were assessed by a third-party blind method outside the research team
Hubei	doctor decides that by envelope method	Not stated
Hubei	Statistical experts use R software to generate random results and put them into numbered envelopes in sequence. After the	open label

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Hubei	Subjects will be assigned to the western medicine treatment group and the TCM treatment group on a 1:1 basis by simple	N/A
Guangdong	Random Numbers Generated by Random Functions	Not stated
Zhejiang	The patients were randomly allocated on a 1:4 ratio((control group:experimental group)) to receive by a computer-	#BLANK
Shanghai Hubei	The procplan program of SAS software was used to generate random numbers and randomly them divided into two groups namely the	Not stated
Guangdong	The research center conducted computer-generated block randomization (block size =8)	Not stated
Hubei	#NOTREQ	Quadruple (Participant, Care Provider, Investigator, Outcomes Assessor)
Guangdong	The Doll's clinical case random table was adopted by the special scientific research assistant, and the method was as follows: the	none
Zhejiang	This study plans to collect 20 subjects, according to the order of the study subjects into the group numbering, using statistical software to	Not stated
Zhejiang	This study plans to collect 100 subjects, according to the order of the study subjects into the group numbering, using statistical	Not stated
Guangdong	Envelope Random Method by Attending Doctor	Not stated

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Shaanxi	The statistician uses the SAS software proc plan program to generate the random sequence.	Open-label
Hubei	The subjects were stratified according to the respiratory support conditions, and then randomly assigned to the test group and the control	It is a double-blind study. Subjects, researchers (all participants in this study, including but not limited to those who are responsible for screening subjects, follow-up and evaluating outcome and compliance with
Jiangxi	A specific researcher is responsible for generating a random sequence using the statistical software SAS 9.1	Open label
Guangdong Henan Sichuan Heilongjiang Shandong	program. Eligible patients Use the central randomization system	Open label
Jiangxi	Randomly enter a group according to the time sequence of subjects' entering the group and the pre-determined random	Open label
Beijing	#NOTREQ	Double (Participant, Care Provider)
Hubei	A static random sequence will be formed by GPHCM and placed on a clinical information collection platform to apply for the	Open label
Hubei	Random numbers generated by computers	Not stated
Hubei	Patients were randomly divided into two groups by the nurse's random envelope method	Open label
Hunan	Subjects were grouped by the random sequence generated according to the random number table by the staff of the Clinical Trial	NA

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Shanghai	Stratified block randomization	Not stated
Guangdong	#NOTREQ	Single (Participant)
Beijing	The researchers in the project team used EXCEL to generate a table of random Numbers, which were divided into two groups	Single blind
Guangdong	#NOTREQ	Quadruple (Participant, Care Provider, Investigator, Outcomes Assessor)
Chongqing	#NOTREQ	Quadruple (Participant, Care Provider, Investigator, Outcomes Assessor)
Fujian	#NOTREQ	None (Open Label)
Guangdong	SAS software will be used to generate random number by stratified block randomization and the length of block will be 2 or 4	Not stated
Hubei	Third parties that do not participate in the study use computer software Excel to generate random sequences and custody	Not stated
Hubei	#NOTREQ	Triple (Participant, Investigator, Outcomes Assessor)
Hubei	Eligible patients were randomly assigned to the Nintedanib group and the control group	Not stated

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Guangdong	SPSS25.0 was used to generate 56 random Numbers in the range of 1-100 (fixed number of seed is 200 000)	Double blind
Anhui Hubei Zhejiang	#NOTREQ	None (Open Label)
Anhui Hubei Zhejiang	#NOTREQ	None (Open Label)
Hubei	Random number table method	Open label
Hubei	The third party uses statistical software to generate random number.	Not stated
Hubei	#NOTREQ	Double (Participant, Investigator)
Zhejiang	The computer assigns random Numbers	Not stated
Henan	Stata 12.0 software will be used to generate random codes by randomization method, and the test vaccine and the control vaccine will	Double blind
Jiangsu Hubei	Meng hongdao, a data statistician, used the software to prepare a random number table, which was used to divide subjects	Open label
Guangdong	Randomized parallel control （Grouped by a computer table of random numbers）	Not stated

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Jiangsu	#NOTREQ	Quadruple (Participant, Care Provider, Investigator, Outcomes Assessor)
Hubei	SAS 9.4 software was used to generate random sequences by statisticians from China Center for Evidence-Based Traditional	Not stated
Hubei	The researchers did randomize the volunteers.Cluster randomization was used in Huangshang Group Simple	#BLANK
Hubei	The researchers used a random number table to randomly divide the patients into three groups, namely the control group, the 1.5ATA	Not stated
Shanghai	The researchers in the project team used EXCEL to generate a table of random Numbers, which were divided into two groups	Not stated
Hubei	#NOTREQ	Single (Outcomes Assessor)
Hubei	Centralized block randomization method was accessed.Subjects are assigned to treatment group and control group on a 1:1	Double blind
Hubei	Implementation of the randomized method through the central randomized system of clinical research	Double blind
Hubei	SAS software was used to randomize the grouping and generate a random number table	Not stated
Heilongjiang	The third party uses a random number table for randomization	Open label

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Shaanxi	#NOTREQ	None (Open Label)
Beijing	Using block randomization, all selected cases are divided into test group and control group, and the block size is 6. Using SAS	Not stated
Sichuan Shanghai Hubei	The random number generated by a computer.	Not stated
Shanghai Hubei	The statistician applies a random sequence number automatically generated by a random number generator (IBM Corp., Armonk, NY	Participants and Liu-zi-jue trainers are unable to be blind about group assignments due to the specific intervention, but the trainers will not know the assessment of outcomes. To reduce the risk of bias, evaluators, data
Hubei	SAS software was used to randomize the grouping and generate a random number table	#BLANK
Hubei	The random sequence will be generated by computer.	Not stated
Henan	Using Stata 12.0 software to generate random codes by randomization, the study vaccine and the placebo were randomly assigned to	Not stated
Hubei	stratified block randomization	Not stated
Guangdong Hubei InnerMongolia	A randomization scheme that generates a list of 160-case randomization sequence utilizing the stratified block randomization method by	double-blind
Hubei	Block randomization was adopted, and the time of admission was used as the compatibility factor, block size is four, by a ratio of 1:1	Double blind

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Guangdong	The enrolled patients will be allocated into routine treatment group and WMT group by the block randomization method	Not stated
Shangnai Guangdong Tianjin Heilongjiang Beijing	In this study, disease severity (mild, moderate, severe) and age (< 60 years old, ≥60 years old) were stratified and randomized	#BLANK
Hebei	#NOTREQ	Double (Participant, Investigator)
Jiangsu	#NOTREQ	Double (Participant, Outcomes Assessor)
Heilongjiang	Randomly grouped by the system	Not stated
Hubei	Statistical professionals generate random number tables according to the clinical research central random system (network	Double blind
Liaoning Beijing Hunan	Randomized with envelopes	Not stated
Zhejiang	Statistical professionals use a computer to generate a table of random numbers, and all patients are numbered by the doctor in	Not stated
Sichuan	#NOTREQ	Quadruple (Participant, Care Provider, Investigator, Outcomes Assessor)
Hubei Jiangsu	Random allocations are generated by the use of an Excel random number table at the design stage and are based on the calculated	Open label

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Guangdong	The random program was provided by the statistics professionals of the school of public health of Fudan University. Stratified (with the	Not stated
Hubei	This study uses the interactive web response system(IWRS) to random,in order to ensure that the allocation is hidden the	Double blind
Shanghai Hubei	The statistician applies a random sequence number automatically generated by a random number generator (IBM Corp., Armonk, NY	Participants and Liu-zi-jue trainers are unable to be blind about group assignments due to the specific intervention, but the trainers will not know the assessment of outcomes. To reduce the risk of bias, evaluators, data
Shanghai	Use the block random method to formulate a randomization plan.	#BLANK
Shanghai	#NOTREQ	Quadruple (Participant, Care Provider, Investigator, Outcomes Assessor)
Zhejiang	The third party uses a random number table for randomization	Not stated
Chongqing	#NOTREQ	Double (Participant, Investigator)
Anhui	#NOTREQ	#NOTREQ
Zhejiang Guangxi	SAS statistical software (version 9.4) is used by an unblinded randomization statistician to generate a randomized blinding code	Not stated
Hunan	#NOTREQ	Double (Participant, Investigator)

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Sichuan	#NOTREQ	Quadruple (Participant, Care Provider, Investigator, Outcomes Assessor)
Beijing	#NOTREQ	Single (Participant)
Beijing	#NOTREQ	Single (Participant)
Beijing	#NOTREQ	Triple (Participant, Care Provider, Investigator)
Hubei	With the aid of SAS9.3 statistical analysis system, a randomized plan of subjects was generated, and the ratio of patients in the two groups	Doubel blinding
Shanghai	Random Numbers generated by statistical software	Double blind
Hunan	#NOTREQ	Quadruple (Participant, Care Provider, Investigator, Outcomes Assessor)
Jiangsu	#NOTREQ	Triple (Participant, Investigator, Outcomes Assessor)
Shanghai	#NOTREQ	Quadruple (Participant, Care Provider, Investigator, Outcomes Assessor)
Jiangsu	#NOTREQ	Triple (Participant, Care Provider, Investigator)

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Hubei	Use the statistical software to generate a random number table	#BLANK
Jiangsu	A block random design is adopted. Each random block contains several random numbers, and each random number corresponds to three	Double blind
Shanghai	The statistician will assign codes according to the order in which the subjects are enrolled.	Double blind
Hebei	#NOTREQ	Double (Participant, Investigator)
Hubei Beijing	Adopting the clinical experiment management method of central randomization system, by dialing the telephone number	#BLANK
Hubei Beijing	Using center randomized clinical trial management system, the method of by call the randomization center (interactive voice response	Not stated
Jiangsu	#NOTREQ	Quadruple (Participant, Care Provider, Investigator, Outcomes Assessor)
Beijing Shanghai	#NOTREQ	None (Open Label)
Hubei	In this experiment, the random distribution table of cases was compiled by spss22.0 statistical software. according to the random	Not stated
Jiangsu	Randomized statisticians use SAS version 9.4 or above to generate random blinding code. In each age and dose group, subjects were divided	Double blind

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Guangdong	#NOTREQ	Quadruple (Participant, Care Provider, Investigator, Outcomes Assessor)
Zhejiang	SAS statistical software (version 9.4) is used by an unblinded randomization statistician to generate a randomized blinding code	Double blind
Hubei	The group of patients will be determined by drawing lots from the nurse in charge	Not stated
Jiangsu	Randomized statisticians use SAS version 9.4 or above to generate random blinding code. In each age and dose group, subjects were divided	Double blind
Jiangsu	#NOTREQ	Double (Participant, Investigator)
Jiangsu	SAS 9.4 software is used to carry out randomization according to the stratified block randomization method.	Double blind
Shanghai	#NOTREQ	Quadruple (Participant, Care Provider, Investigator, Outcomes Assessor)
Jiangsu	#NOTREQ	Quadruple (Participant, Care Provider, Investigator, Outcomes Assessor)
Jiangsu	#NOTREQ	Triple (Participant, Care Provider, Investigator)
Jiangsu	#NOTREQ	Triple (Participant, Care Provider, Investigator)

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Jiangsu	The randomization program is completed by the randomization team.	Double blind
Hunan	This trial adopts a stratified block randomization method, stratified by center (participating country), age (18-59 years old, 60 years	This study adopts a double-blind design. Randomization statisticians and other blind-editing personnel will perform vaccine blinding, that is, paste the printed vaccine label on the designated position of each
Shaanxi	#NOTREQ	Single (Participant)
Shanghai	Computer software produces random sequences	Not stated
Beijing	#NOTREQ	Single (Participant)
Shanxi	Randomize according to the cluster random method with SAS 9.4 software.	Not stated
Shanxi	Randomize according to the block random method with SAS 9.4 software.	Not stated
Shanxi	Randomize according to the block random method with SAS 9.4 software.	Not stated
Guangxi Shanxi	SAS statistical software (version 9.4) is used by an unblinded randomization statistician to generate a randomized blinding code	Blind method for subjects.
Hebei Heilongjiang	Statistical professionals use a computer to generate a table of random numbers, and all patients are numbered by the doctor in	Open-label

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Hebei Heilongjiang	Statistical professionals use a computer to generate a table of random numbers, and all patients are numbered by the doctor in	Open-label
Hebei Heilongjiang	Statistical professionals use a computer to generate a table of random numbers, and all patients are numbered by the doctor in	Not stated
Zhejiang	In this study, a randomized block method was used, with each subject assigned to the trial group or placebo group determined by a randomized	Double blind
Jiangsu	#NOTREQ	Triple (Participant, Care Provider, Investigator)
Hebei	The statistical staff at evidence-based medicine center of Tianjin University of traditional Chinese medicine used SAS software to	Not stated
Hebei	Statistical professionals use a computer to generate a table of random numbers, and all patients are numbered by the doctor in	Not stated
Hubei	Generated by Computer software	Double blind
Beijing Guangdong Jiangsu Yunnan	#NOTREQ	Single (Participant)
Shanghai	SAS software will be used to generate random codes by randomization method. The ratio of experimental group and control group is 1:1	Open-label
Zhejiang	#NOTREQ	None (Open Label)

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Jiangsu	#NOTREQ	Triple (Participant, Care Provider, Investigator)
Sichuan	Random number table	Not stated
Jiangsu	#NOTREQ	Triple (Participant, Investigator, Outcomes Assessor)
Guangdong	For the planned 2-dose and 1-dose youth (18-59 years) and elder (>=60 years) groups, block randomization is performed by the	Double blind
Guangdong	As a dose-escalating study, it is planned to adopt six groups according to age (youth and elder) and dosage (10 ug, 25ug and	Double blind
Guangdong	The randomization will be carried out in the Clinical Trail Unit of the First Affiliated Hospital, Sun Yat-sen University The subjects	N/A
Jiangsu	#NOTREQ	Quadruple (Participant, Care Provider, Investigator, Outcomes Assessor)
Henan	#NOTREQ	Double (Participant, Investigator)
Henan	#NOTREQ	None (Open Label)
Zhejiang	Randomized blind bottom was generated by block randomization using SAS statistical software (version 9.4 or above) by randomized	Double blind

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Hebei	#NOTREQ	Double (Participant, Investigator)
Shanghai Sichuan Shanxi	The researchers use SAS software for block randomization	Not stated
Hubei Henan Guangdong	The researchers use SAS software for block randomization	Not stated
Hebei	Central block randomization method	Not stated
Liaoning	#NOTREQ	Double (Investigator, Outcomes Assessor)
Jiangsu	#NOTREQ	Quadruple (Participant, Care Provider, Investigator, Outcomes Assessor)
Jiangsu	#NOTREQ	Triple (Participant, Investigator, Outcomes Assessor)
Jiangsu	SAS 9.4 software is used to carry out randomization according to the stratified block randomization method.	Not stated
Shanghai	SAS software will be used to generate random codes by randomization method. The length of blockl is 10	Not stated
Guangdong	The central random method is used to intervene according to the allocation plan. The treatment plan received by each research	Not stated

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Shanxi Sichuan	#NOTREQ	Quadruple (Participant, Care Provider, Investigator, Outcomes Assessor)
Anhui	#NOTREQ	Triple (Participant, Care Provider, Investigator)
Shandong	#NOTREQ	None (Open Label)
Hu'nan	#NOTREQ	Quadruple (Participant, Care Provider, Investigator, Outcomes Assessor)
He'nan	#NOTREQ	Triple (Participant, Investigator, Outcomes Assessor)
He'nan	#NOTREQ	Triple (Participant, Investigator, Outcomes Assessor)
Jiangsu	#NOTREQ	Double (Participant, Investigator)
Hu'nan	#NOTREQ	Double (Participant, Investigator)
Hu'nan	#NOTREQ	Double (Participant, Investigator)
Jiangsu	#NOTREQ	Double (Participant, Investigator)

For peer review only

Jiangsu	#NOTREQ	Triple (Participant, Care Provider, Investigator)
Hu'nan	The method of randomization and blinding was realized by vaccine editing, and the random table was generated by SAS	Not stated
Hubei Jiangsu	Random Stratified random number table	Not stated
Jiangsu	The clinical research methodology team of Guangdong Hospital of traditional Chinese medicine generated random	Open-label
Jiangsu	The central randomization method is used to intervene according to the allocation plan. The treatment plan received by each research	Not stated
Jiangsu	SAS 9.4 software was used to implement the cluster randomization by statisticians from China Center for Evidence-Based	Open label
Yunnan	Randomization codes will be generated by an unblinded statistician responsible for randomization using SAS statistical software (version	Not stated
Shanxi	Randomization codes will be generated by an unblinded statistitian responsible for randomization using SAS statistical software (version	Not stated
Chongqing	Randomization list will have been generated by a Statistician using a computer and will be properly kept by a Designated Study	#BLANK
Zhejiang	The randomization statistician used SAS statistical software (version 9.4 or above) to randomize each dose group of the adult	Not stated

For peer review only

Guangxi Yunnan	Non-blind randomization statisticians use SAS statistical software (version 9.4) to generate randomized blind bottoms using block	Not stated
Hubei	SAS was used to generate random sequences	Not stated
Shanghai	#NOTREQ	None (Open Label)
Shanghai	#NOTREQ	None (Open Label)
Hubei	Clinical researchers will get a random sequence number which is automatically generated by a random number generator	Not stated
Jiangsu	#NOTREQ	None (Open Label)
Hubei	SAS was used to generate random sequences	Not stated
Jiangsu	#NOTREQ	Triple (Participant, Investigator, Outcomes Assessor)
Beijing	#NOTREQ	None (Open Label)
Guangdong He'nan Hubei	#NOTREQ	Single (Outcomes Assessor)

For peer review only

Hebei Heilongjiang	#NOTREQ	Single (Participant)
Jiangsu	#NOTREQ	Double (Participant, Investigator)
Hu'nan	#NOTREQ	Quadruple (Participant, Care Provider, Investigator, Outcomes Assessor)
Hubei	Dr. Le Shengjong, a member of the project team, divided the subjects into two groups (20 in each group) according to the random numbers	Not stated
Hubei	Random number table method.	Open label
Hu'nan	Random and blind methods were realized through vaccine coding, and SAS software was used to generate random tables. The	Double blind
Beijing	The statistician uses the random block method to generate random sequences.	Not stated
He'nan	In this study, subjects were randomly assigned to the experimental or control group.The randomization table was randomly	Open-label
Shanghai	The statistician used SPSS statistical software to randomly assign 50 numbers into 2 groups by randomization	Not stated
Hu'nan	#NOTREQ	None (Open Label)

For peer review only

Guangxi	The randomization codes will be generated by the randomization statistician using the block randomization method	Not stated
Guangdong	#NOTREQ	None (Open Label)
Sichuan	The randomization program is completed by the randomization team.	Double blind
Shanghai	This study is an open randomized controlled study design. Professional medical statisticians use SAS?9.4 statistical software to	Open-label
Shanxi	#NOTREQ	Double (Participant, Investigator)
Jiangsu	#NOTREQ	Triple (Participant, Investigator, Outcomes Assessor)
Guizhou	#NOTREQ	None (Open Label)
Beijing	#NOTREQ	Double (Participant, Investigator)
Guangdong	#NOTREQ	Double (Participant, Investigator)
Yunnan	Non-blind randomization statisticians use SAS statistical software (version 9.4) to generate randomized blind bottoms using block	Not stated

For peer review only

Shanghai	#NOTREQ	Double (Participant, Investigator)
Beijing	#NOTREQ	Triple (Participant, Investigator, Outcomes Assessor)
Beijing	#NOTREQ	Triple (Participant, Investigator, Outcomes Assessor)
Hubei	The patients were randomly divided into two groups by the central random system at a 1:1 ratio.	Not stated
Sichuan	The trial used a block randomization method, stratified by center and age (18-59 years, ≥60 years). The randomization of	#BLANK
Beijing	Central Random System Block Random	Not stated
Yunnan	The randomization codes will be generated by the randomization statistician using the block randomization method	Not stated
Jiangsu	This study was divided into intervention group and control group by random number method. Patients with COVID-19 were	#BLANK
Jiangsu	The randomization program is completed by the randomization team	Not stated
Jiangsu	#NOTREQ	None (Open Label)

For peer review only

Shanghai	#NOTREQ	Triple (Participant, Care Provider, Investigator)
Shanghai Beijing	#NOTREQ	None (Open Label)
Anhui Beijing Hebei Hu'nan	#NOTREQ	Double (Participant, Investigator)
Hebei Beijing Hunan Chongqing Fujian	#NOTREQ	Triple (Participant, Investigator, Outcomes Assessor)
Shaanxi	A researcher used a random number table method to generate a random sequence, with odd numbers entering the control group	#BLANK
Hunan	Generate random sequences with SPSS software.	#BLANK

For peer review only

Participants (Patient or Suspected/Close Contacts or Healthy/Other)
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For peer review only

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For peer review only

Inclusion Criteria

Inclusion criteria: 1. Adult aged >=18years old;
2. Patients infected with new coronavirus confirmed by PCR
3. In the state of no oxygen at rest, the patient's SPO2<=94% or the oxygenation index is less than 300mmHg.

Inclusion Criteria:

Adult
PCR confirmed COVID-19 infection
Symptoms developed more than 7 days

Inclusion Criteria:

Confirmed COVID-19 infection case in term of laboratory evidence;
80 years ≥ age ≥ 14years;
Within 72 hours after the onset of abnormalities shown by Chest radiology or several symptoms

Inclusion criteria: 1. Male or female over 18 years of age;
2. Novel coronavirus infection is confirmed by pathogenic detection;
3. The diagnosis of severe coronavirus pneumonia will have to meet at least one of the following criteria:
(1) Respiratory distress, RR>30 times/minute
(2) In the state of no oxygen at rest, the patient's SPO2<=93%
(3) Oxygenation Index (PaO2/FiO2)<=300 mmHg (1mmHg=0.133kPa);
(4) Respiratory failure requiring mechanical

Inclusion criteria: (1) 18â€“65 years of age;
(2) Diagnosed as mild to moderate COVID-19;
(3) Be willing to sign informed consent;
(4) The clinical assessment and condition of the subject does not influence the completion of the study.

Inclusion Criteria:

The participants were diagnosed as COVID-19 pneumonia, according to the notice on printing and distributing the diagnosis and treatment plan of pneumonia with new coronavirus infection (trial version 4 or update version) made by National Health Commission of the People's Republic of

Inclusion criteria: 1) Aged >= 18 years;
2) Meet the diagnostic criteria of severe pneumonia patients with new coronavirus;
3) The informed consent form.

Inclusion Criteria:

14-70 years old
2019 novel coronavirus pneumonia (severe type)
Subjects voluntarily participate in the clinical trial and sign the informed consent

Inclusion Criteria:

In sputum, throat swab, lower respiratory tract secretion, blood and other samples, the nucleic acid of the novel coronavirus was positive, or the sequencing of the virus gene was highly homologous with the known novel coronavirus

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Inclusion Criteria:

Age ≥18 years at time of signing Informed Consent Form
Laboratory (RT-PCR) confirmed COVID-19.
Lung involvement confirmed with chest imaging

Inclusion Criteria:

Age ≥18 years at time of signing Informed Consent Form
Laboratory (RT-PCR) confirmed COVID-19.
Lung involvement confirmed with chest imaging

Inclusion criteria: 1. 2019-nCoV infected patients treated at Affiliated Zhongsnan Hospital of Dalian University and Zhongnan Hospital of Wuhan University from January 2020 to December 2020
2. nCoV nucleic acid positive;
3. Aged >=18 years;
4. Patients understand the purpose of the trial, voluntarily sign informed consent, and agree to complete the rectification research process

Inclusion criteria: 1. Those who meet the diagnostic criteria for suspected cases in the diagnosis and treatment scheme;
2. Over 18 years old (including 18 years old), male or female;
3. At the time of enrollment, the patient had any of the main symptoms (fever, cough, fatigue);
4. Voluntarily sign a written informed consent before the study begins.

1. Patients with confirmed new coronavirus-infected pneumonia;
2. Above 18 years old (inclusive);
3. At the time of enrollment, the patient had any of the main symptoms (fever, cough, fatigue);
4. Voluntarily sign written informed consent.

Inclusion criteria: 1. aged 18 to 60 years
2. to be confirmed with a exposure history of diagnosed population within last 14 days;
3. with fever, positive results of blood routine examination and positive findings in CT examination, any two in three items met.

Inclusion criteria: 1) Pneumonia patients diagnosed as new coronavirus infection;
2) The type of pneumonia was judged as severe and critical;
3) Informed consent.

Inclusion criteria: 1) patients diagnosed as Pneumonia caused by new coronavirus infection (2019-nCoV);
2) The classification was judged as common type;
3) Informed consent.

Inclusion criteria: 1. patients diagnosed as Pneumonia caused by new coronavirus infection (2019-nCoV);

2. virus turned negative after treatment;

3. aged 18-70 years old;

4. patients with clear consciousness;

Inclusion criteria: 1. patients diagnosed as Pneumonia caused by new coronavirus infection (2019-nCoV);
2. virus turned negative after treatment;
3. aged 18-70 years old;
4. patients with clear consciousness
5. Signing the informed consent form.

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Inclusion criteria: 1. patients diagnosed as Pneumonia caused by new coronavirus infection (2019-nCoV);The classification was judged as common type;
2. aged 18-70 years old;
3. patients with clear consciousness;
4. Signing the informed consent form.

Inclusion criteria: 1. patients diagnosed as Pneumonia caused by new coronavirus infection (2019-nCoV);
2. virus turned negative after treatment;
3. Patients diagnosed with pulmonary fibrosis;
4. aged 18-70 years old;
5. patients with clear consciousness;
6. Signing the informed consent form.

Inclusion criteria: 1. patients diagnosed as Pneumonia caused by new coronavirus infection (2019-nCoV);
2. virus turned negative after treatment;
3. aged 18-70 years old;
4. patients with clear consciousness
5. Signing the informed consent form.

Inclusion Criteria:

1)2019-nCoV nucleic acid test was positive. 2)CT of the lung conformed to the manifestation of viral pneumonia.

Inclusion Criteria:

2019-nCoV nucleic acid test was positive.
CT of the lung conformed to the manifestation of viral pneumonia.

Inclusion Criteria:

1.Meet the definition of severe pneumonia(Comply with any of the followings):

Shortness of breath RR>30 bpm;

Inclusion criteria: (1) aged 18 to 70 years;
(2) Light and heavy patients with new coronavirus infection confirmed by etiology.

Inclusion criteria: (1) Conforming to the diagnostic criteria of suspected cases (Chiese medicine observation period) in the fourth edition of pneumonia diagnosis and treatment program for new coronavirus infection issued by the national health and health commission;
(2) Age between 14 and 80 years old;
(3) Patients informed and agreed to participate in the study.

Inclusion criteria: (1) Patients who meet the diagnostic criteria of the common type of confirmed cases (the initial stage of clinical treatment of traditional Chinese medicine) or the severe cases of confirmed cases (the middle stage of clinical treatment of traditional Chinese medicine);
(2) The cases of common type (the initial stage of clinical treatment of traditional Chinese medicine) belong to the type of cold-dampness stagnant lung;
(3) The confirmed cases were severe (in the

Inclusion criteria: 1. Adult aged >= 18 years old;
2. Patients with unexplained viral pneumonia that have been clinically diagnosed or patients with novel coronavirus serum antibody (IgM or IgG) positive or patients with novel coronavirus infection confirmed by PCR when no rapid diagnostic kit has been developed;
3. The interval between the onset of symptoms and randomized is within 7 days. The onset of symptoms is mainly based on fever. If there is no fever, cough or other related

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Inclusion criteria: 1. Aged 18-65 years;
2. been Confirmed with 2019-nCoV pneumonia, hospitalized or will be hospitalized;
3. Sign the ICF;
4. no more than 10 days from the occurrence of relevant clinical symptoms to the diagnosis been confirmed.

Inclusion criteria: 1. 18 to 75 years of age, male or female, willing to sign informed consent;
2. Tested positive for novel coronavirus infection after the onset of symptoms using a real time polymerase chain reaction (RT-PCR)-based diagnostic assay;
3. At least 2 viral nucleic acid tests are still positive under current antiviral therapy;
4. No difficulty in swallowing the pills;
5. Willing to abide by the protocol

Inclusion criteria: 1. Aged 18 to 75 years male or female, willing to sign the informed consent;
2. Tested positive for novel coronavirus infection after the onset of symptoms using a real time polymerase chain
reaction (RT-PCR)-based diagnostic assay;
3. Enrollment and initiation of study drug treatment <=96 hours after onset of symptoms;
4. No difficulty in swallowing the pills;
5. Willing to abide by the protocol

Inclusion criteria: Patients with suspected and confirmed diagnosis of 2019-nCoV Pneumonia Version 4.0 (published at 2020.01.27) of the National Health Commission

Inclusion criteria: Patients with novel coronavirus pneumonia who agreed to participate in this trial and signed the informed consent form.

Inclusion criteria: On January 27, 2020, the general office of the national health commission and the office of the state administration of traditional Chinese medicine issued the pneumonia diagnosis and treatment plan for new coronavirus infection (trial version 4).
1. patients with severe 2019-ncov according to the clinical stage met any of the following criteria:
(1) Respiratory distress, RR>=30 times/min;
(2) In resting state, oxygen saturation is less than 93%;
(3) Partial

Inclusion Criteria:

Age:18~55 years old, unlimited gender.
Laboratory (RT-PCR) confirmed infection with 2019-nCoV and accompanied with clinical manifestations

Inclusion Criteria:

≥ 18 years old;
Diagnosed as serious or critical SARI (according to the 4th version of Diagnosis and Clinical management of 2019-nCoV infected pneumonia);

Inclusion criteria: On January 27, 2020, the general office of the national health commission and the office of the state administration of traditional Chinese medicine issued the pneumonia diagnosis and treatment plan for new coronavirus infection (trial version 4).
Only patients with severe and critical 2019-ncov were included in this study.
1. Patients with severe 2019-ncov according to the clinical stage met any of the following criteria:
(1) Respiratory distress, RR >= 30 times/min;

Inclusion criteria: o be enrolled, all the following criteria must be met:
1) the confirmed patient (or legal guardian) agrees to participate in the study and signs the informed consent;
2) Aged 18-65 years, gender not limited;
3) the time interval between symptom onset and random enrollment was within 8 days (symptom onset was mainly determined by the time of fever);
4) real-time fluorescence rt-pcr detection of respiratory tract or blood samples in patients with positive

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Inclusion criteria: 1. Aged >= 18 years and <= 75 years;
2. Patients clinically diagnosed as novel coronavirus infection; or with positive serum antibodies (IgM or IgG); or with novel coronavirus infection confirmed by PCR;
3. Severe/critical patients.

Inclusion criteria: (1) Comply with the diagnostic criteria of "Pneumonitis Diagnosis and Treatment Scheme for New Coronavirus Infection (Trial Version 5)";
(2) The classification was judged as common and severe type;
(3) Having one or more symptoms of fever, cough and fatigue when entering the group;
(4) The diagnosis time does not exceed 48 hours;
(5) Aged >= 18 years

Inclusion criteria: 1. Suspected and confirmed cases of pneumonia in accordance with the above new signs of coronavirus infection and the standard of clinical common diagnosis;
2. Aged 18-65 years old;
3. Signing the general informed consent form.

Inclusion criteria: 1. Conforming to the diagnostic criteria for the above close contacts;
2. Aged between 18 and 65 years old;
3. Signing the generalized informed consent form. (The intervention objects is incorporated in the clinical study as desired).

Inclusion criteria: 1. Aged 18-75 years old;
2. Patients with novel coronavirus infection pneumonia were confirmed by RT-PCR and clinical symptoms. The diagnostic criteria refer to "Pneumonitis Diagnosis and Treatment Scheme for Novel Coronavirus Infection (Trial Version 5)";
3. Patients with newly diagnosed respiratory system discomfort who have been hospitalized (the diagnosis time of respiratory system discomfort shall not exceed 7 days);
4. Women and

Inclusion criteria: 1. Patients with confirmed new coronavirus-infected pneumonia;
2. Above 18 years old (inclusive);
3. Voluntarily sign written informed consent.

Inclusion Criteria:

The participants were diagnosed as COVID-19 pneumonia, according to the notice on printing and distributing the diagnosis and treatment plan of pneumonia with new coronavirus infection (trial version 4 or update version) made by National Health Commission of the People's Republic of

Inclusion criteria: 1. Diagnosed as novel coronavirus pneumonia (NCP) Patient:
1) Basis of diagnostic criteria: "Notice on Printing and Distributing Pneumonia Diagnosis and Treatment Plan for New Coronavirus Infection (Trial Implementation Fourth Edition)" (National Health Office Medical Letter C2020377)
2019-nCoV diagnosis of pneumonia:
(1) Epidemiological history: A. Travel history or residence history in Wuhan area or other areas with continuous local case

Inclusion criteria: 1. Age over 18 years old;
2. Real-time fluorescent RT-PCR for respiratory or blood specimens to detect novel coronavirus nucleic acid positive;
3. The sequence of virus genes in respiratory or blood samples was highly homologous with the known novel coronavirus;
4. A common or severe type of new type of coronavirus pneumonia was diagnosed. The common patients diagnosed with novel coronavirus pneumonia that have fever, respiratory

Inclusion criteria: 1. Adults (defined as age >= 18 years);
2. Patients with new type of coronavirus infection confirmed by PCR / serum antibodies;
3. The time interval between symptom onset and random enrollment is within 10 days.The onset of symptoms is mainly based on fever. If there is no fever, cough or other related symptoms can be used;
4. Imaging confirmed pneumonia;
5. In the state of no oxygen at rest, the patient's blood oxygen saturation SPO2 <=

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Inclusion criteria: 1. Eligible patients were adults ((aged 18-80 years);
2. Were identified as laboratory-confirmed 2019-nCoV infection;
3. Those who need intubation in ICU, and with no severe hypoxemia.

Inclusion criteria: Patients with novel coronavirus infection who agreed to participate in this trial and signed the informed consent form. The diagnose criteria was according to Diagnostic and treatment protocol of COVID-19 pneumonia published by Chinese General Office of National Health Committee(6th and further edition).

Inclusion criteria: (All the following criteria are met before being selected):
1. Aged >= 18 years;
2. Meet all the following criteria (refer to confirmed cases in the 5th edition of the diagnosis and treatment plan):
(1) Epidemiological history;
(2) Clinical manifestations (according to any of the following 2): fever; normal or decreased white blood cell count or reduced lymphocyte count in the early stages of onset; multiple small patchy shadows and interstitial changes in early chest

Inclusion criteria: 1. Aged >=18 to 70 years;
2. Diagnosed 2019-nCoV infected patients by viral nucleic acid,or although the test was negative and even not be detected, the reseachers judged it to be a novel coronavirus pneumonia;
3. Understand and agree to participate in this study and sign informed consent;
4. According to the guideline of clinical classification of novel coronavirus pneumonia (2019-nCoV) diagnosis and treatment plan (trial version fifth), the general

Inclusion criteria: Conforming to the diagnostic standard of COVID-19; Participants in the first and second trials need to meet the diagnostic criteria of traditional Chinese medicine for epidemic diseases; Participants in the third trial need to meet the diagnostic criteria of deficiency of qi in lung and spleen of epidemic diseases.

Inclusion criteria: 1. Patients who were confirmed diagnosis of 2019-ncov virus pneumonia, according to the "pneumonia diagnosis and treatment program for novel coronavirus infection (trial version 5)",including clinical symptoms, pulmonary CT manifestations and positive nucleic acid test;
2. Inpatients 18 years old (including 18), regardless of gender;
3. Willing to participate in this clinical trial and sign the informed consent

Inclusion criteria: 1. 18 years old <= age <= 60 years old at the time of screening;
2. Pharyngeal swabs, alveolar lavage fluid, or sputum specimens during screening showed that 2019-nCoV virus nucleic acid test results were positive for at least two of nCov-EP, nCov-NP, and nCovORF1ab, in line with the release of a new coronavirus infection by the National Health and Medical Commission Diagnostic criteria for confirmed cases in the pneumonia diagnosis and treatment plan (trial version

Inclusion criteria: 1. The patient signed an informed consent form to participate in the study of convalescent plasma therapy;
2.The age of the patient is 18 or more than18 years old;
3. COVID-19 patients diagnosed by PCR;
4. Nucleic acid positive within 72 hours before blood transfusion;
5. Pneumonia confirmed by imaging;
6. The clinical symptoms reach the standard of severe or critical.
Severe patients meet any of the following:
a) respiratory

Inclusion criteria: 1. Aged 18- 80 years old;
2. Pneumonia patients with new coronavirus infection were confirmed to be positive by RT-PCR and clinical manifestations. The diagnosis standard refers to the diagnosis and treatment plan for pneumonia with new coronavirus infection (trial version 5);
3. Patients with newly diagnosed respiratory system discomfort who have been hospitalized (the diagnosis time of respiratory system discomfort shall not exceed 7 days);
4

Inclusion criteria: 1. Informed consent offered;
2. Confirmed infection with 2019-nCoV by RT-PCR;
3. Compliance with the intervention of study.

For peer review only

Inclusion criteria: Subjects who meet the following conditions can become subjects of this study:
1. Sign written informed consent. Adolescents or patients who cannot sign informed consent must obtain informed consent from the guardian;
2. Chinese who are >= 18 years of age;
3. It complies with the diagnostic criteria for NCP diagnosis of common patients, that is, the "Pneumonitis Diagnosis and Treatment Scheme for New Coronavirus Infection (Trial Fifth Edition)"

Inclusion criteria: Subjects who meet the following conditions can become subjects of this study:
1. Sign written informed consent. Adolescents or patients who cannot sign informed consent must obtain informed consent from the guardian;
2. Chinese aged >=18 years;
3. It complies with the NCP Critical and Critical Diagnostic Standards, namely "Pneumonitis Diagnosis and Treatment Scheme for Novel Coronavirus Infection (Trial Version 5), Combined with the

Inclusion criteria: (1) Comply with the diagnostic criteria for general type COVID-19 in the "Diagnosis and Treatment Program for COVID-19"(trial version 5th).
(2) Agree to participate in the study and the patient or legal guardian signs the informed consent form.

Inclusion criteria: 1. The patients who were diagnosed with the common type of NCP (including severe risk factors) and severe cases of new coronavirus pneumonia;
2. Aged 18 to 85 years;
3. IL-6 elevated (using Elisa method, using the same company kit);
4. Patients or authorized family members volunteered to participate in this study and signed informed consent.
Definition of Novel Coronavirus Pneumonia (NCP) clinical cases:
1. Regular patients with

Inclusion criteria: 1. Aged 18-75 years male or female;
2. Body weight 40-100 kg and BMI >= 18kg/m2;
3. Been Confirmed with commom 2019-nCoV pneumonia inpatient;
4. With fever, respiratory tract and other symptoms, imaging shows pneumonia;
5. 2019-nCoV nucleic acid test was positive by pathogenic detection within 72 hours;
6. Have undergone surgical sterilization or take effective contraception during the trial;
7. Written the informed consent

Inclusion criteria: Severe patients (according to any of the following): respiratory distress (RR> 30 beats / min); at rest, means oxygen saturation <= 93%; arterial oxygen pressure (PaO2) / oxygen concentration (FiO2) < = 300 mmHg

Inclusion criteria: 1. Patients with confirmed new coronavirus-infected pneumonia;
2. Above 18 years old (inclusive);
3. Voluntarily sign written informed consent.

Inclusion criteria: 1. Hospitalized patients with novel coronavirus pneumonia confirmed by pathogenic detection
2. Meet any one of the criteria for severe type:
1) Respiratory distress: RP >= 30/min;
2) At rest, the oxygen saturation <= 93%;
3) Arterial partial pressure of oxygen (PaO2)/Fraction of inspiration (FiO2)(Oxygenation Index, P/F) <= 300mmHg.
3. Aged 18 to 75 years;
4. Signed informed consent

Inclusion criteria: 1) Aged >= 18 years;
2) In line with the diagnostic criteria of "new coronavirus pneumonia diagnosis program", the patient was diagnosed as pneumonia infected by the new coronavirus;
3) Severe pneumonia index (PSI) class for III ~ IV;
4) The course of symptoms should be less than or equal to 8 days;
5) Volunteers and comply with the hospital regulations, can cooperate with the completion of the prescribed inspection;
6) Voluntary

Inclusion criteria: 1) Aged >= 18 years;
2) In line with the diagnostic criteria of "new coronavirus pneumonia diagnosis program", the patient was diagnosed as pneumonia infected by the new coronavirus;
3) Severe pneumonia index (PSI) class for I ~ II;
4) Hospitalized for:
Fever: axillary temperature >= 36.7 degree C, oral temperature >= 38.0 degree C, Anal temperature >= 38.6 degree C;
respiratory rate > 24 times / min or

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Inclusion criteria: 1. Meet the diagnostic criteria of the "New Coronavirus Pneumonia Diagnosis and Treatment Scheme", patients with pneumonia confirmed by new coronavirus infection;
2. Pneumonia Severity Index (PSI) grade is I ~ II;
3. Aged 18-80 years;
4. Those who have signed the informed consent voluntarily.

Inclusion criteria: 1. General/severe/critical patients who meet the diagnostic criteria of new coronavirus pneumonia;
2. Patients whose viral nucleic acid has turned negative after treatment;
3. Patients who are characterized by respiratory symptoms such as cough, shortness of breath, chest tightness or corresponding chest imaging features;
4. Patients who meet TCM syndrome;
5. Patients who voluntarily participate in the study and sign informed

Inclusion criteria: (1) Comply with the standards of diagnosis and classification of western medicine, and the standards of syndromes of traditional Chinese medicine.
(2) Aged 18-80 years.
(3) Those who voluntarily participated in this research and signed written informed consent.

Inclusion Criteria:

1. Male or female, aged at 18 years-65 years old
2. Pneumonia that is judged by chest radiograph or computed tomography
3. Laboratory confirmation of NCP infection by reverse-transcription polymerase chain reaction(RT-

Inclusion criteria: 1. 18 years old <= age <= 60 years old when screening, not limited to men and women;
2. Who are in close contact with suspected cases, confirmed cases, and positive test of new coronavirus nucleic acid, and those who have not taken effective protection, that is, close contacts;
3. Study subjects are willing to be randomly assigned to any designated treatment group;
4. Must agree to not participate in another study of study drugs until day 24 of

Inclusion criteria: 1. 2019 adult patients with new-type coronavirus pneumonia diagnosed with 2019-nCoV infection by PCR according to the "Pneumonitis Diagnosis and Treatment Protocol for New-type Coronavirus Infection (Trial Version 5)"
2. The absolute value of lymphocytes <0.6x10^9/L;
3. Severe respiratory failure does not exceed 48 hours and requires ICU treatment. Among them, severe respiratory failure is defined as PaO2 / FiO2 <200 mmHg and requires positive

Inclusion criteria: (1) Those who meet the diagnosis of new coronavirus pneumonia and have clinical classification of low and medium;
(2) Those who have been diagnosed with TCM as epidemic-closed lungs;
(3) Inpatients aged 18 to 75 years, regardless of gender;
(4) Voluntarily receive treatment with the drug and sign informed consent. The informed consent process complies with relevant GCP regulations

Inclusion criteria: 1. Age is not limited;
2. Clinical diagnosis is in accordance with the "Notice on issuing a new type of coronavirus pneumonia diagnosis and treatment program (Fifth edition)" on the diagnosis of a new type of coronavirus infection;
3. potable decoction;
4. no honeysuckle allergy;
5. child-bearing age female subjects pregnancy test negative person;
6. child-bearing age female subjects pregnancy test positive person needs targeted

Inclusion criteria: 1. Patients who meet the requirements of the New Coronavirus Infected Pneumonia Diagnosis and Treatment Scheme (revision of Trial Version 5);
2. Aged 18-75 years male or female;
3. Serious patient meet one of the following:
(1) respiratory distress wiht respiratory rate >=30 breaths/min;
(2) oxygen saturation<=93% on quiet status;
(3) Arterial partial pressure of oxygen(PaO2)/oxygen concentration<=300mmHg;
4. critically ill patients

Inclusion Criteria:

CT image is characteristic of viral pneumonia;
2019-ncov infection (positive nucleic acid test) is confirmed by pathogenic test;
In compliance with the 2019-nCoV infection severe pneumonia diagnosis standard (according to the

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Inclusion Criteria:

Aged between 18 and 80 years (Including 18and 80years, male or female).
One of them:

Inclusion criteria: Subjects who meet the all of the following criterion to be elegiable for this study:
1. Meet the requirements of the "New Coronavirus Infected Pneumonia Diagnosis and Treatment Scheme (revision of Trial Version 5)";
2. Aged 18 to 65 years male or female;
3. The interval from the onset of symptoms to enrollment is within 7days: onset of symptoms is defined as onset of at least one systematic symtom(such as fever(temperature>=37.3 degree C) or

Inclusion criteria: Participants with 18 to 75 years of age who have been diagnosed with NCP by nucleic acid testing and meet the clinical classification of severe NCP referring to the clinical classification standard in the General Office of the National Health and Health Commission-Novel Coronavirus Pneumonia Diagnosis and Treatment Program (Trial Version 4).

Inclusion criteria: 1. Severe patients diagnosed with novel coronavirus pneumonia (COVID-19);
2. Aged 35 years old to 74 years old;
3. Patients or authorized family members volunteered to participate in this study and signed informed consent.
Note: Case definition: Severe NCP: Refer to the "New Coronavirus Pneumonia Diagnosis and Treatment Plan (Fifth Edition)" formulated by the National Health Commission.

Inclusion criteria: (1) aged >=18 years old;
(2) real-time fluorescence rt-pcr of respiratory or blood samples presents the positive nucleic acid of COVID-19, or the viral gene sequencing of respiratory or blood samples was highly homologous with the known COVID-19.
(3) patients diagnosed with novel coronavirus meet the diagnostic criteria of the latest clinical guidelines for novel coronavirus issued by the world health organization (WHO) on January 28, 2020 and the

Inclusion criteria: 1) common type pneumonia patients with confirmed new coronavirus infection;
2) aged 18 to 75 years old;
3) voluntary signing of written informed consent.

Inclusion criteria: 1. Subjects have signed informed consent, agree not to participate in other clinical studies within 30 days from the first administration of the research drug to the last administration.
2. Aged >= 18 and <= 75 years.
3. Met the diagnostic criteria for COVID-19 (fifth edition, China).
4. SOFA score: 2 to 13 points.
COVID-19 clinical stratifications:
1. Mild (n = 200): The clinical symptoms are mild or asymptomatic, and no pneumonia is seen on imaging; but

Inclusion criteria: 1. Aged 18 years old and above;
2. Meet the novel coronavirus pneumonia (COVID-19) diagnostic criteria. The upper and lower respiratory tract RT-PCR confirmed that 2019-nCoV nucleic acid positive, chest CT imaging examination could be used in conjunction;
3. SaO2/SpO2=94% under indoor air, or PaO2/FiO2<300mgHg (for severe type patients);
4. Sign informed consent;
5. Do not participate in the clinical study of other study drugs within 28 days

Inclusion criteria: 1. patients with novel coronavirus pneumonia confirmed by pathogenic detection;
2. Fever, respiratory tract and other symptoms, the change of pneumonia can be seen in imaging;
3. Aged 18 to 80 years;
4. Signed informed consent.

Inclusion Criteria:

age more than 18 years old
accord with the clinical diagnosis and/ or etiological diagnosis diagnostic criteria of Novel coronavirus pneumonia (COVID-19)

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Inclusion criteria (I) Discharged patients fulfilling the diagnostic criteria of COVID-19 (China Diagnosis and Treatment Protocol for COVID-19 (Trial Version 7); (II) COVID-19 RNA in respiratory specimens or blood specimens of patients is negative (>2 times), assayed by real-time fluorescent polymerase chain reaction test (RT-PCR); (III) Pulmonary CT scans within 7 days showed that there were still unabsorbed inflammation or pulmonary fibrosis in the lungs; (IV) Age 18-70

Inclusion criteria: 1. Age 18 to 75 Yearsmale or female subjects
2. Diagnosed with COVID-19,according to "New Coronavirus Infected Pneumonia Diagnosis and Treatment Scheme (Pilot Sixth Edition);
3. Sever COVID-19, except shock and ICU care with organ failure;
4. Antimalarial drugs such as chloroquine and hydroxychloroquine sulfate were not used within 3 months ;
5. Agreed not to participate in other clinical trials in the period of 30 days from first

Inclusion criteria: 1. Aged >=18 years at time of signing Informed Consent Form;
2. Patients with confirmed cases meeting the following criteria
(1) Epidemiology history;
(2) Clinical manifestations (epidemiological history and in accordance with any of the following 2 items; no clear epidemiology history, conforms to 3 items in clinical manifestations):
Fever and / or respiratory symptoms;
In the early stage of the disease, the total leukocyte count was normal or

Inclusion criteria: Patients aged 18 or older, and meet the diagnostic criteria of Diagnosis and Treatment Scheme of Novel Coronavirus Infected Pneumonia published by the National Health Commission.
Criteria for diagnosis (meet all the following criteria):
1. With epidemiological history;
2. Clinical manifestations (meet any 2 of the following): fever, normal or decreased white blood cell count or lymphopenia in the early stage of onset, and Chest radiology in the early

Inclusion criteria: 1. Meet the diagnostic criteria for suspected and confirmed cases (including mild, general, and severe) in the Diagnosis and Treatment of Pneumonia of New Coronavirus Pneumonia (Trial Version 5);
2. Meet the TCM diagnostic criteria for each stage of the novel coronavirus pneumonia;
3. Aged 18-75 years-old men or women;
4. Patients who volunteer to participate in this study and sign informed consent

Inclusion criteria: 1. Meet the diagnostic criteria forconfirmed cases in the Diagnosis and Treatment of Pneumonia of New Coronavirus Pneumonia (Trial Version 5);
2. The acute stage is complicated by sepsis, respiratory failure, ARDS and so on. The patients are with multiple organ damage when improved and tended to recover after organ support and other treatments;
3. Meet the TCM diagnostic criteria for the recovery period;
4. Aged 18-75 years-old men or

Inclusion criteria: 1. Diagnosed Novel coronavirus pneumonia patients or clinically diagnosed NCP patients in Hubei Province;
2. Aged 18 to 65 years;
3. The time interval between symptom onset and randomization was within 7 days (the onset of symptom was mainly determined by the time of fever);
4. Sign the written informed consent before the study.

Inclusion criteria: cured patients of 2019 novel coronavirus.

(1) Aged 18-65 years;
(2) Virus nucleic acid test is positive;
(3) Lung CT showed multiple patchy, lumpy ground glass opacity(GGO), consolidation or other typical manifestations in both lungs .

Inclusion criteria: 1. Patients or his/her legal representatives agreed to participate the study and signed the informed consent.
2. Those diagnosed with mild or moderate novel coronavirus pneumonia (NCP) or those with severe NCP but can take medicines orally or via stomach tube. The confirmed patients were defined as those NCP suspects with one of the following pathogenic evidences:
1) The novel coronavirus in respiratory or blood specimens were confirmed by real

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Inclusion criteria: 1. Aged 18 to 60 years old;
2. Inpatients who meet the diagnostic criteria for mild, general and neonatal pneumonia in western medicine, and early and recovery stages of traditional Chinese medicine;
3. Understand and agree to participate in this study and sign the informed consent.

Inclusion criteria: 1) Male and female, 18-70 years old;
2) Severe patients (in any case: respiratory distress, RR >= 30 times / min; in resting state, oxygen saturation <= 93%; PaO2 / FiO2 <= 300MMHG);
3) Within 12 days after illness onset;
4) Agree to sign the informed consent;
5) Able to ingest, absorb and tolerate oral medication;
6) Agree to collect clinical samples;
7) Consent to contraceptive measures for sex within 7 days of the last oral

Inclusion criteria: 1. Aged >=60 years;
2. Patients confirmed with novel coronavirus pneumonia. The Diagnostic criteria refer to "Pneumonitis Diagnosis and Treatment Scheme for Novel Coronavirus Infection (Trial Version 5)";
3. Those without broad-spectrum antibacterial, antifungal, and antiviral treatment;
4. Imaging revealed pneumonia in patients with a diagnosis of pathogen: influenza, CAP/HAP bacterial infection, and fungal infection;
5. Participants signed

Inclusion criteria: 1. Male or female, 18-95 years old;
2. 2019 new coronavirus nucleic acid test positive;
3. Diagnosed as moderate to severe cases of new coronavirus pneumonia in 2019, which meets the following clinical manifestations:
4. With fever and respiratory symptoms, imaging shows pneumonia;
5. A severe case diagnosed with 2019 new coronavirus pneumonia, that is, any of the following:
(1) Increased breathing rate (>=30 beats / min), difficulty breathing

Inclusion criteria: 1. Patients withmild and moderate novel coronavirus pneumonia (COVID-19) diagnosed refers to the diagnosis and treatment plan for pneumonia with new coronavirus infection (trial version 5);
2. Patient cough score > 1;
3. Aged 18- 80 years old;
4. The subjects (including male subjects) had no pregnancy, sperm donor, egg donor for nearly half a year, and were willing to take effective contraception from the first dose until three months after the last

Inclusion criteria: 1. The age at the time of signing the informed consent is >= 18 years old and <= 75 years old, both male and female;
2. The change of typical viral pneumonia in lung was detected by RT-PCR test of novel coronavirus nucleic acids or lung CT;
3. SaO2 / SpO2 <= 94% or pa02 / fi02 ratio < 300mghg at room temperature at admission;
4. Onset <=12 days;
5. Willing to participate in the trial and randomly assigned to any treatment group;
6

Inclusion criteria: 1. Aged>= 18, male or female;
2. Fever, axillary temperature > 37.3 degrees;
3. Consistent with the clinical diagnosis of COVID-19 mild, normal, severe case;
4. Patients who sign informed consent form freely and voluntarily.

Inclusion criteria: (1) Meet NCP critical diagnosis criteria;
(2) Volunteer to participate in this study, cooperate with examination and treatment;
(3) The selected patients were basically stable, conscious and cooperative;
(4) No serious mental disorder;
(6) Aged between 20-80 years.

Inclusion criteria: (1) Adult aged >=18 years and agrees to sign the informed consent;
(2) Laboratory-confirmed 2019-nCoV infection by real-time RT-PCR;
(3) Chest imaging (CT) confirmed lung damage: multiple small plaques and stromal changes in the lungs, which were obvious in the outer lung, or multiple ground-glass shadows and infiltration shadows in both lungs, the changes may not be present in mild patients;
(4) Hospitalized patients with fever (axillary

Inclusion criteria: 1. The patients who were diagnosed with the common type of NCP (including severe risk factors) and severe cases of new coronavirus pneumonia;
2. Aged 18 to 85 years;
3. IL-1ÃŸ elevated (using Elisa method, using the same company kit);
4. Patients or authorized family members volunteered to participate in this study and signed informed consent;
Definition of Novel Coronavirus Pneumonia (NCP) clinical cases:
1. Regular patients with

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Inclusion criteria: (1) Patients with mild who are in compliance with the "Pneumonitis Diagnosis and Treatment Program for COVID-19(Trial Fifth Edition)" (released on 2020.2.4);
(2) Aged more than 18 years old;
(3) Informed consent of the patient.

Inclusion criteria: (1) In line with ethics, sign a written informed consent form;
(2) Patients with positive detection of new coronavirus nucleic acid;
(3) Chest CT confirmed pulmonary lesions;
(4) Hospitalized patients with fever or respiratory symptoms;
(5) Patients who did not participate in other clinical studies within three months.

Inclusion criteria: 1. Patients aged 15-80 years,diagnosed with mild, common, and severe novel coronavirus pneumonia in accordance with the diagnostic criteria for the diagnosis and treatment of novel coronavirus pneumonia issued by the National Health Commission (trial version 5);
2. White blood cell Count < 15.0 x 10^9/L and Lymphocyte Count < 0.8 x10^9/L.

Inclusion criteria: 1. Aged 18 to 70 years old, inpatients, male or female;
2. Patients with severe novel coronavirus infection: According to the "Pneumonitis Diagnosis and Treatment Guideline for the Novel Coronavirus Infection (Trial Version 5)", clinically diagnosed cases (suspected cases with pneumonia imaging features) or suspected cases.
Severe patients must also meet any of the following:
1) Respiratory distress, RR>=30 times / minute
2) In the resting state, the

Inclusion criteria: Patients aged >=18 years with novel coronavirus pneumonia who agreed to participate in this trial and signed the informed consent.

Inclusion Criteria:

Age ≥18 years at time of signing Informed Consent Form.
Laboratory (RT-PCR) confirmed infection with CoVID-19.
Hospitalised with Fever(>36.7°C, axilla or oral temperature > 38.0 °C or >38.6°C tympanic or rectal)

Inclusion criteria: 1. Voluntarily sign the Consent Form;
2. Male or female aged 18 and over;
3. According to the "Diagnosis and Treatment Scheme for Pneumonia of Novel coronavirus pneumonia Infection-Trial Fifth Edition", inpatient diagnosed with Novel coronavirus pneumonia diagnosed and clinical classification of ordinary type: Inpatients with fever (underarm temperature >= 37.0 degree C), respiratory tract, etc. Imaging shows pneumonia

Inclusion criteria: pneumoniaits were diagnosed by chest CT findings and positive nucleic acid tests

Inclusion criteria: (1) Age 18 to 80 years old (including threshold), sex unlimited, inpatient
(2) Respiratory specimen were positive for COVID-19 nucleic acid by RT-PCR;
(3) Clinical classification is mild or moderate type (with symptoms of fever, respiratory tract, imaging findings of pneumonia);
(4) Hospitalization for the following reasons:Fever (axillary temperature >= 36.7 degrees C, or mouth temperature >= 38.0 degrees C, or anal or ear temperature >= 38.6 degrees C)

Inclusion criteria: (1) RT-PCR detection of the respiratory or blood samples shows positive nucleic acid of the novel coronavirus;
(2) Gene sequence of the virus in respiratory or blood samples are highly homologous to the known coronaviruses;
(3) The CT images of chest shows radiographic features of pneumonia;
(4) Patients with severe or critical conditions;
(5) Aged >=18, male or female;
(6) The subjects should volunteer to join the study, sign the informed

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Inclusion criteria: 1. Meet the requirements of the "New Coronavirus Infected Pneumonia Diagnosis and Treatment Scheme (revision of Trial Version 5)";
2. Aged 18 to 65 years male or female;
3. The interval from the onset of symptoms to enrollment is within 7days: onset of symptoms is defined as onset of at least one systematic symtom(such as fever(temperature>=37.3 degree C) or shivering headache hypodynamia muscle pain or joint pain) or respiratory symptom(such as

Inclusion criteria: 1. Aged 18 to 80 years (inclusive);
2. People who had ineffective protection, i.e.:
1) living, studying, working together with, or having close contacts with confirmed cases and positive testers, such as working at close distances or sharing the same classroom or living in the same house;
2) had close contacts with medical personnel, family members or people who have similar close contact during the diagnosis, treatment, and visits, such as visiting or staying in a

Inclusion criteria: (1) Patients in accordance with the diagnostic criteria of "Quick reference guide for diagnosis and treatment of novel coronavirus pneumonia(2019-nCoV)(Standard Edition)" and "Diagnostic and treatment protocols of novel coronavirus pneumonia(Trial version 5)" issued by National Health Commission;
(2) Patients in accordance with the TCM syndrome differentiation standard;
(3) Aged 18 to 85 years;
(4) Before the trial begins, patients or their legal

Inclusion criteria: 1. Confirmed and suspected cases of pneumonia from novel coronavirus infection;
2. Meet the diagnostic criteria for community - acquired pneumonia;
3. Clinical classification is "heavy" or "critical";
4. Aged 18 to 75 years;
5. Obtain informed consent.

Inclusion Criteria:

Laboratory diagnosis:

Respiratory specimen is positive for SARS-CoV-2 nucleic acid by RT-PCR; OR

Inclusion criteria: 1. Aged 18-75 years old;
2. RT-PCR positive for 2019-nCoV or Confirmed lung involvement with chest imaging;
3. The clinical symptoms are slight, and there is no obvious manifestation of pneumonia (mild disease) or clinical symptoms such as fever and respiratory tract in imaging. The manifestation of pneumonia (common type) can be seen in imaging;
4. <=7 days since onset of the disease;
5. Willingness of study participant to accept

Inclusion criteria: Suspected cases of neo-coronary pneumonia (outside Hubei province), clinically diagnosed cases (in Hubei province), and light / general confirmed cases, severe/critical confirmed cases, aged 18-70 years, each center enrolls cases in a competitive manner.
The diagnostic criteria for the new coronavirus pneumonia conform to the diagnostic criteria issued by the National Health and Medical Commission for the diagnosis and treatment of pneumonia in the new

Inclusion criteria: 1. aged 18-70 years male or female;
2. 2019 new coronavirus nucleic acid test positive;
3. meet the national diagnosis criteria;
4. subjects onset within 8 days (severe cases can be delayed to 14 days);
5. subjects or agency personnel agree to sign the informed consent;
6. subjects who can take oral medication or who receive oral medication;
7. subjects agree to collect clinical samples;
8. female subjects of child-bearing age are not

Inclusion criteria: Patients with mild/common novel coronavirus pneumonia who agreed to participate in this trial and signed the informed consent form

Inclusion criteria: 1. Age between 18 and 75 years old (including threshold), regardless of gender;
2. The patient under isolation observation is diagnosed with mild or common type of 2019-nCoV acute respiratory disease and can cooperate to complete corresponding psychological intervention;
3. The total score of Hamilton depression scale (HAMD17) and Hamilton anxiety scale (HAMA14) at baseline is both at least 7;
4. The patient has sufficient compliance to complete

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Inclusion criteria: 1) age 18-80 years old, regardless of gender: 2) the diagnosis criteria of severe nCoV pneumonia in line with the 6th edition of China's new coronavirus pneumonia diagnosis scheme: a.B. Real-time fluorescence rt-pcr of respiratory tract specimens or blood samples was used to detect positive nCoV nucleic acid;3) critical cases of nCoV pneumonia: meet one of the following criteria: a. respiratory failure and mechanical ventilation required;B. shock;C. Combined

Inclusion criteria: Subjects who meet the following conditions can become subjects of this study:
1. Will sign written informed consent. Adolescents or patients who cannot sign informed consent must obtain informed consent from the guardian;
2. Chinese aged >=18 years;
3. In accordance with NCP criteria for severe and critical illness, namely "Pneumonitis Diagnosis and Treatment Scheme for Novel Coronavirus Infection (Trial Version 6)". Combined with the following

Inclusion criteria: 1. Aged 18-80 years;
2. Within 1 week of onset;
3. Meet the diagnostic criteria of common, severe and critical NCP, according to "Pneumonitis Diagnosis and Treatment Scheme for Novel Coronavirus Pneumonia (Trial Version 6)".
4. Sign informed consent.

Inclusion criteria: Patients are eligible for admission if they meet all of the following criteria within 5-7 days of their first ICU admission.
1) Aged 18-75 years;
2) The patient meets the ARDS diagnostic standard (Berlin standard) and needs intubation ventilator-assisted or ECMO-assisted treatment;
3) The primary disease caused by 2019-nCoV virus infection;
4) Imaging data suggest bilateral lung lesions;
5) Voluntarily participate in clinical research; fully understand

Inclusion criteria: 1. Aged 18-70 years old;
2. Meet the diagnostic criteria for confirmed cases in the diagnosis and treatment plan of new coronavirus-infected pneumonia (trial version 5); and the clinical symptoms before admission or screening must meet the following conditions:
Fever (armpit temperature >=37.3 degree C, or mouth temperature >=38.0 degree C, or anal or ear temperature >=38.6 degree C) <=7 days; and breathing rate> 24 beats / min or cough (at least one

Inclusion criteria: (1) All patients were diagnosed with a new type of coronavirus pneumonia;
(2) Clear mind, cooperation and communication;
(3) Men and women aged 18 and 80 years;
(4) informed consent has been signed and is willing to be assessed.

Inclusion criteria: 1. Aged>=18 years old;
2. Diagnosed as severe and critical patients with novel coronavirus pneumonia;
3. Being treated in the ICU.

Inclusion criteria: Subjects who met all of the following conditions entered the trial:
1. Male or female patients aged 16 to 75 years;
2. willing to sign informed consent;
3. Based on the criteria for suspected cases of COVID-19 infection, sputum, throat swabs, lower respiratory tract secretions and other specimens were tested for positive COVID-19 nucleic acid by real-time fluorescent RT-PCR; or viral gene sequencing was performed with known COVID-19 highly

Inclusion criteria: inclusion of case criteria (pneumonia diagnosis and treatment protocol for COVID-19 (trial 6th edition))
1. Common type patient: with fever, respiratory tract and other symptoms, and imaging findings of pneumonia.
2. Severe patients: patients who meet any of the following criteria
(1) respiratory distress, RR>=30 times/min;
(2) in resting state, oxygen saturation is less than 93%;
(3) partial arterial oxygen pressure (PaO2)/oxygen absorption concentration

Inclusion criteria: (1) Those who meet the diagnosis of new coronavirus pneumonia and have clinical classification of low and medium;
(2) Inpatients aged 18 to 75 years;
(3) Voluntarily receive treatment with the drug and sign informed consent. The informed consent process complies with relevant GCP regulations.

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Inclusion criteria: 1. Aged 18-60 years old;
2. Patients with clinical symptoms, signs, auxiliary detection combined with 1 or more nucleic acid test to confirm the diagnosis of new coronavirus pneumonia;
3. Voluntarily sign the informed consent to comply with the requirements of the program.

Inclusion criteria: 1) The confirmed patient (or legal guardian) agrees to participate in the study and signs the informed consent form;
2) aged 18 to 65 years;
3) Real-time fluorescent RT-PCR of respiratory specimens or blood specimens to detect patients with new type of coronavirus positive;
4) Patients diagnosed as severe and critically ill and with rapid disease progression according to the "Diagnosis and Treatment Program for Pneumonia of New Coronavirus Infection

Inclusion criteria: 1. Sign the written informed consent;
2. Aged 18-80 years old;
3. Meet the diagnostic criteria for novel coronavirus infection(trial version sixth), confirmed by combining with the epidemiological history, clinical manifestations and etiology examination;
4. Course of disease within 14 days after onset.

Inclusion criteria: (1) Comply with the diagnostic criteria for COVID-19 in the "Diagnosis and Treatment Program for pneumonia caused by novel coronavirus infection (Trial Version 5)", meet the the criteria of severe and critical type;
(2) Agree to participate in the study and the patient or legal guardian signs the informed consent form.

Inclusion Criteria:

Male or female, aged at 18 years (including) -75 years old
Hospitalized
Laboratory confirmation of SARS-CoV-2 infection by reverse-transcription polymerase chain

Inclusion Criteria:

Male or female, 18 years old ≤ age ≤ 75years old;
CT image is characteristic of 2019 novel coronavirus pneumonia;
Laboratory confirmation of 2019-nCoV infection by reverse-transcription polymerase chain reaction

Inclusion criteria: 1. Patients discharged from severe or critical new coronavirus pneumonia;
2. Adults over 18 years of age who signed the informed consent;
3. Study participants are willing to accept random assignment to any designated treatment group;
4. Prior to the completion of this study, participants must agree not to participate in another study.

Inclusion criteria: 1) The confirmed patient (or legal guardian) agrees to participate in the study and signs the informed consent form;
2) aged 18 to 65 years;
3) The time interval between the onset of symptoms and random enrollment is within 8 days (the onset of symptoms is mainly based on the occurrence of fever);
4) Real-time fluorescent RT-PCR of respiratory specimens or blood specimens to detect patients with 2019-nCoV nucleic acid positive;
5) Patients were diagnosed

Inclusion criteria: (1) Aged >=18 years;
(2) Novel coronavirus pneumonia patients diagnosed by pathogenic testing;
(3) The patient himself participated in the study voluntarily, agreed and signed the informed consent.

Inclusion criteria: 1. Female patients aged >=18 years who were of childbearing years were excluded from pregnancy and agreed to use non-drug contraceptive measures during the study period;
2. SARS-CoV-2 nucleic acid test positive;
3. Lung CT examination showed evaluable lesions;
4. Meet the diagnostic criteria for severe or critical illness in the diagnosis and treatment program for new coronavirus pneumonia (trial version 6);
5. Subjects voluntarily

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Inclusion criteria: 1. Aged 18 to 80 years;
2. Patients with novel coronavirus pneumonia who were diagnosed as general type or severe type according to the western medicine classification general type, severe type and Chinese medicine classification criteria;
3. Understand and agree to participate in the study and sign the informed consent.

Inclusion criteria: (1) Aged 18 years or older;
(2) Voluntarily signed informed consent;
(3) Hospitalized patients diagnosed as COVID-19.

Inclusion criteria: 1. Patients with confirmed new coronavirus-infected pneumonia, clinical classification is ordinary or heavy;
2. TCM syndrome differentiation is wind-heat syndrome;
3. Aged between 18-75 years old, regardless of gender;
4. Voluntarily sign written informed consent.

Inclusion criteria: 1. Aged 18 to 75 years old;
2. Pneumonia patients with new coronavirus infection were confirmed to be positive by RT-PCR and clinical manifestations. The diagnosis standard refers to the current diagnosis and treatment plan for pneumonia with new coronavirus infection;
3. Patients with newly diagnosed respiratory system discomfort who have been hospitalized (the diagnosis time of respiratory system discomfort shall not exceed 7 days);
4

Inclusion criteria: 1. novel coronavirus pneumonia patients with nutritional malnutrition diagnosed by nutritional assessment;
2. enteral nutrition support treatment for at least 5 days;
3. No use of Fat emulsion taboo;

Inclusion criteria: Patients meet the diagnostic criteria "Diagnosis and treatment of novel coronavirus pneumonia (trial version fifth)"for suspected cases outside Hubei Province: combined with the following epidemiological and clinical manifestations: epidemiological history
1. Patients with travel or residence history in Wuhan and surrounding areas or other communities with case reports within 14 days before the onset of the disease;
2. Patients with a novel coronavirus infection

Inclusion criteria: 1. Aged 18 to 85 years old;
2. It conforms to the diagnostic criteria of severe and critical type in "Diagnosis and Treatment Plan for Pneumonia Infected in novel coronavirus (6th Edition)";
3. Sign the informed consent form.

(1) Comply with the diagnostic criteria for general type COVID-19 in the "Diagnosis and Treatment Program for COVID-19"(trial version 6th).
(2) Aged >= 18 years.
(3) Agree to participate in the trial, and the patient, the legal guardian or the person in charge of the medical institution signed the informed consent through paper signature, electronic signature of

Inclusion criteria: 1. First diagnosis of novel coronavirus infection
2. Epidemiological history Travel history or residence history of Wuhan City within two weeks before the onset of illness; or patients who have had fever from Wuhan with respiratory symptoms within 14 days before the onset of illness, or have cluster Onset;
3. Clinical manifestations:
(1) fever;
(2) with the following imaging characteristics of pneumonia: early lesions are limited, showing patchy

Inclusion criteria: Patients undergoing painless gastroscopy in our hospital.

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Inclusion criteria: (1) Comply with the diagnostic criteria for general type COVID-19 in the "Diagnosis and Treatment Program for COVID-19"(trial version 6th);
(2) Patients =18 years of age with tracheal intubation;
(3) Agree to participate in the trial, and the patient, the legal guardian or the person in charge of the medical institution signed the informed consent through paper signature, electronic signature of mobile phone software, or voice authorization

Inclusion criteria: 1. New-type coronavirus infection is diagnosed as mild or common (diagnosed according to the "Notice on the Issuance of a New Coronavirus-Infected Pneumonia Diagnosis and Treatment Plan (Trial Implementation of the Sixth Edition)") or patients with poor immune function (cured discharged patients but peripheral hemolymph Cell count and classification are abnormal).
2. Aged 18-65 years;
3. Sign informed consent

Inclusion criteria of Cohort 1 #4 is adapted from the Flu guideline in China (2018) and community acquired pneumonia (CAP) guideline in China (2016)

Cohort 1:

Inclusion criteria: 1. Aged 18 to 70 years old;
2. Patients with pneumonia infected by new coronavirus: confirmed cases in compliance with the Diagnosis and Treatment Plan for Pneumonia Infected by New Coronavirus (Sixth Edition);
3. Confirmed cases who can be transfused into anti-SARS-CoV-2 virus inactivated plasma after fully assessing patients condition;
4. Cases who score 3 or 4 according to 6-grade scale;
5. The subjects and/or their legal guardians

Inclusion criteria: 1. The patient signs the written informed consent;
2. Aged 18-80 years;
3. the novel coronavirus novel coronavirus infection diagnostic criteria for pneumonia, namely, the new coronavirus pneumonia diagnosis and treatment plan (trial version sixth). It conforms to any of the following:
1) in case of shortness of breath, RR >= 30 times / min;
2) in case of resting state, oxygen saturation <= 93%;
3) in case of arterial partial pressure of oxygen (PaO2) /

Inclusion criteria: 1. Aged 18 to 75 years men and women;
2. Inpatients with novel coronavirus(SARS-CoV-2) infection confirmed by PCR;
3. Agree and sign the informed consent form.

Inclusion criteria: 1. Aged 18 to 65 years old, both male and female;
2. The eligible subjects (male and female) with fertility must agree to take effective contraceptive measures from the beginning of self-sufficiency with their partners to 6 months after drug withdrawal;
3. Novel coronavirus pneumonia diagnosis novel coronavirus pneumonia novel coronavirus pneumonia diagnosis and treatment plan (latest edition) is confirmed;
4. Drugs without chloroquine and its

Inclusion criteria: 1. Aged 18-80 years;

2. Meet the diagnostic criteria of "New Coronavirus Infected Pneumonia Sixth Edition";

3. Blood pressure 90-150 / 60-100mmHg, respiratory rate 12-20 beats / min, heart rate 60-100 beats / min;

4. Extremity muscle strength examination is more than 5-grade;

Inclusion criteria: 1) confirmed or suspected cases of new coronavirus pneumonia;
2) be able to understand the content of this study, and have the ability of normal communication and understanding;
3) voluntarily participate in this study and obtain written informed consent from the guardian or the patient.

Inclusion criteria: (1) Aged 20-45 years old;
(2) Understand and willing to participate in this clinical trial and provide signed informed consent.

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Inclusion criteria: 1. Eligible patients were adults ((aged 18-80 years);
2. Were identified as laboratory-confirmed COVID-19 infection;
3. Those who need intubation in ICU, and with no severe hypoxemia.

Inclusion criteria: 1) confirmed or suspected cases of new coronavirus pneumonia;
2) be able to understand the content of this study, and have the ability of normal communication and understanding;
3) voluntarily participate in this study and obtain written informed consent from the guardian or the patient.

Inclusion criteria: 1. Real-time fluorescent RT-PCR detection of new coronavirus nucleic acid positive, or viral gene sequencing is highly homologous with known new coronavirus;
2. Confirmed as a novel coronavirus pneumonia common and severe patients, meets the diagnostic criteria of the "novel coronavirus pneumonia diagnosis and treatment plan (trial implementation of the sixth edition)"
3. aged 18 to 65 years;
4. Learn about the significance of the study

Inclusion criteria: (1) Patients aged from 18 to 75 years old;
(2) Mild patients with confirmed COVID-19 according to the diagnostic criteria of "novel coronavirus Infection Diagnosis and Treatment Plan (Trial 6th Edition)" issued by the National Health and Safety Commission;
(3) Patients and their families voluntarily participated in the study and signed an informed consent form.

Inclusion criteria: (1) diagnosed as a mild and common type of the new coronavirus pneumonia, meeting the diagnostic criteria of the new coronavirus pneumonia diagnosis plan (trial version 6), both male and female;
(2) aged 14 to 70 years old;
(3) body temperature > 37.3 degree C;
(4) understand the nature, significance, possible benefits, inconvenience and potential dangers of the test in detail before the test;
(5) understand the study procedure and voluntarily sign the

Inclusion criteria: 1) Confirmed or clinically diagnosed patients with the novel coronavirus disease;
2) Those who have been identified by Chinese medicine as pestilence stagnant in Lung;
3) Inpatients aged 18 to 75 years male or female;
4) Volunteer to receive treatment under their own scheme and sign informed consent.

Inclusion criteria: 1) Confirmed or clinically diagnosed patients with the novel coronavirus disease;
2) Those who have been identified by Chinese medicine as pestilence stagnant in Lung;
3) Inpatients aged 18 to 75 years male or female;
4) Volunteer to receive treatment under their own scheme and sign informed consent.

Inclusion criteria: 1. Aged >=18 years;
2. Clinically diagnosed patients with COVID-19, including: on the basis of meeting the criteria for suspected cases, one of the following etiology evidence:
(1) real-time fluorescent RT-PCR detection of SARS-Cov-2 nucleic acid samples in respiratory or blood samples;
(2) Sequencing of viral genes in respiratory specimens or blood specimens, highly homologous to known SARS-Cov-2;
3. The interval between the onset of

Inclusion criteria: Patients who are more than 18 years are definitely diagnosed with COVID-19. [That is, the diagnosis of 2019-nCoV-infected pneumonia patients was diagnosed according to the diagnostic criteria for novel coronavirus pneumonia diagnosis and treatment program (trial version 5) issued by the National Health and Health Commission on February 5, 2020.

Inclusion criteria: (1) age: 18-75 years old (including threshold), regardless of gender;
(2) compliance with the pneumonia diagnosis and treatment protocol for novel coronavirus infection - the diagnostic criteria for the trial version 6 (the latest version, which is currently updated to version 6).
(3) the sixth version of the reference treatment scheme meets the common type standard;
(4) no participation in traditional health exercise (ba duan jin, tai chi, wu qin xi or other fitness

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Inclusion criteria: (1) The novel coronavirus was detected at the age of 18~70 years when signing the informed consent; (2) the nucleic acid test of the pharyngeal test, sputum, lower respiratory tract secretions, blood and so on (RT-PCR detection of the new coronavirus nucleic acid positive) and the sequencing of the virus gene, which were highly homologous with the known new coronavirus, and confirmed that the 2019-nCoV was detected. Chest X * ray and CT imaging confirmed pneumonia

Inclusion Criteria:

Patients with mild and severe cases who have been diagnosed with new coronavirus pneumonia according to the "Pneumonitis Diagnosis and Treatment -
Plan for New Coronavirus Infection"

Inclusion criteria: 1. Male or female aged 18-80 years old;
2. Patients with confirmed novel coronavirus-infected pneumonia;
3. Diagnosed as moderate cases of COVID-19 with fever and respiratory symptoms and image showing pneumonia;
4. Voluntarily sign written informed consent.

Inclusion criteria: (1) Aged 18 years or older, male or female;
(2) Real-time fluorescent RT-PCR of pharyngeal swab samples was positive for novel coronavirus nucleic acid;
(3) Clinical classification is common or severe;
(4) Signed informed consent.

Inclusion criteria: 1. Laboratory confirmed infection with 2019-nCoV;
2. Clinical classification is mild or common;
3. Lung involvement confirmed with chest imaging;
4. Hospitalised with: Fever: axilla temperature >= 36.7 degrees C, or Oral temperature >= 38.0 degrees C, or rectal temperature >= 38.6 degrees C.And at least one of Respiratory rate > 24/min Or Cough;
5. TCM syndrome differentiation is phlegm-heat obstructing the lung syndrome;
6. Must agree not

Inclusion criteria: 1. Aged 18 to 75 years;
2. Real-time fluorescence RT-PCR detection of nucleic acid positive of novel coronavirus in respiratory tract specimens or blood preparation;
3. Sequencing of virus genes in respiratory tract specimens or blood preparation, highly homologous to known novel coronavirus ;
4. The patients are diagnosed with common type of novel coronavirus pneumonia according to the Pneumonia Diagnosis and Treatment Program for Novel

Inclusion criteria: 1. Adult patients confirmed by 2019-nCov throat swab or other samples of nucleic acid positive;
2. Patients with clear manifestations of pulmonary exudation on chest imaging;
3. Patients requiring oxygen therapy or mechanical ventilation, ARDS (oxygenation index < 200mmHg) was diagnosed according to the Berlin standard in 2012.

Inclusion criteria: 1. Diagnosed Novel coronavirus pneumonia patients or clinically diagnosed NCP patients in Hubei Province;
2. Aged 18 to 65 years;
3. The time interval between symptom onset and randomization was within 7 days (the onset of symptom was mainly determined by the time of fever);
4. Sign the written informed consent before the study.

Inclusion criteria: Patients with new type of coronavirus pneumonia admitted to the designated hospital of Jinan City have the same diagnostic criteria as "Pneumonitis diagnosis and treatment plan for new type of coronavirus infection (latest version)
during 2020.02-2020.05

Inclusion criteria: (1) aged 18-80 years old;
(2) severe (including critical risk factors) and critical patients with confirmed novel coronavirus pneumonia;
(3) blood CRP and ESR were more than twice higher than normal or TNFa and Il-6 were higher than normal;
(4) sign the informed consent.

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Inclusion criteria: Patients who were diagnosed as COVID-19 by nucleic acid test and were in accordance with the clinical classification of severe or critically illness. (refer to the clinical classification criteria in the pneumonia diagnosis and treatment program of novel coronavirus infection, General Office of the National Health Commission (trial version 4)).

Inclusion Criteria:

Clinically diagnosed with Corona Virus Disease 2019
Increased interleukin-6
Sign the informed consent

Inclusion criteria: Aged 2-65 years, with a BMI of 18-28, who underwent otolaryngological surgery. The patients were ASA grade I-III.

Inclusion Criteria:

Age: 18-80 years old, male and female;
Covid-19 confirmed cases;
Comply with any of the following:

Inclusion criteria: 1. Those who agree to take part in the test and sign the informed consent form voluntarily;
2. Adults aged >=18years old, male or female;
3. The inpatients with mild or common type of COVID-19 confirmed according to the diagnostic criteria "COVID-19 diagnosis and treatment plan - Sixth trial edition" issued by the National Health Commission.
4. Respiratory Specimens (including but not limited to sputum, nasopharyngeal swab and secretion of lower

Inclusion criteria: 1. Those who agree to take part in the test and sign the informed consent form voluntarily;
2. Adult aged >=18years old, male or female;
3. The inpatients with mild or common type of COVID-19 confirmed according to the diagnostic criteria "COVID-19 diagnosis and treatment plan - Sixth trial edition" issued by the National Health Commission.
4. Respiratory Specimens (including but not limited to sputum, nasopharyngeal swab and secretion of lower

Inclusion criteria: 1. The patient signed an informed consent form to participate in the study of convalescent plasma therapy;
2. patient aged >=18 years old;
3. COVID-19 patients diagnosed by PCR;
4. Nucleic acid positive within 72 hours before blood transfusion;
5. Pneumonia confirmed by imaging;
6. Hospitalization for the following reason:
Fever: axillary temperature>=36.7 degree C, or oral temperature>=38.0 degree C, or anal or ear

Inclusion criteria: 1. Aged 18 to 75 years old;
2. The diagnosis of new-type coronavirus pneumonia was confirmed by RT-PCR or gene sequencing according to the "Diagnosis and Treatment Scheme for Pneumonia of New Coronavirus Infection (Trial Version 6 and Updated)", and the clinical type was general (including severe high-risk factors) and severe;
 Note: Patients with common new type of coronavirus pneumonia (including severe high-risk factors); patients with

Inclusion criteria: 1. Novel coronavirus pneumonia is diagnosed met the fifth edition of the national health and Health Committee;
2. Respiratory rate in calm state >=30 times / min; OR Finger pulse oxygen saturation under rest and non oxygen absorption state <=93%; OR PO2/FiO2)<=300mmHg; OR PO2<60mmHg.

Inclusion criteria: 1. Male and female, older than 18 years;
2. Patients with confirmed COVID19;
3. Need to continue the antivirus therapy;
4. Agree to sign the informed consent;
5. Able to ingest, absorb and tolerate oral medication;
6. Agree to collect clinical samples;
7. Consent to contraceptive measures for sex within 7 days of the last oral medication.

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Inclusion criteria: 1. Conforming to the Chinese and Western Medicine Diagnostic Standards of patients with novel coronavirus pneumonia (COVID-19) in recovery period;
2. Aged between 18 and 65 years old;
3. Signing the generalized informed consent form (The intervention objects is incorporated in the clinical study as desired).

Inclusion criteria: 1. Confirmed cases: positive nucleic acid test;
2. Aged 25-65 years;
3. Asexual difference;
4. Clinical stage (light, common type);
5. Immunology standard: peripheral blood lymphocyte was higher than the lower limit of normal;

Inclusion criteria: (1) Patients with Novel coronavirus pneumonia diagnosed according to <diagnosis and treatment scheme for pneumonia of new coronavirus infection (trial version 5)>;
(2) Inpatients aged >18 years old;
(3) Patients Know the informed consent, voluntary participate in this clinical trial and sign informed consent.

Inclusion criteria: 1. Aged between 18 and 75 years;
2. In accordance with NCP criteria for severe and critical illness. Combined with the following epidemiological history, clinical manifestations, and pathogenic examinations to make a comprehensive judgment;
3. Will not participate in other drug clinical trials since the first day of Ulinastatin Injection admission;
4. Will sign written informed consent. Adolescents or patients who cannot sign informed consent must

Inclusion criteria: (1) aged between 18 and 75 years old;
(2) Common type patients with confirmed COVID-19 according to the diagnostic criteria of "novel coronavirus Infection Diagnosis and Treatment Plan (Trial 6th Edition)" issued by the National Health and Safety Commission;
(3) Patients and their families voluntarily participated in the study and signed an informed consent form.

Inclusion Criteria:

Age ≥18 years at time of signing Informed Consent Form;
Those who meet the diagnosis of severe new coronavirus pneumonia with laboratory confirmed infection with CoVID-19;

Inclusion criteria: 1. Patients aged from 18 to 80 years;
2. conformed to epidemic history;
3. fever and respiratory symptoms;
4. normal or decreased leukocyte count in peripheral blood; or decreased lymphocyte count;
5. conformed to imaging features of COVID-19 pneumonia;
6. positive for COVID-19 nucleic acid test or second-generation sequencing of pathogens;
7. serum KL-6 1500U/ml;
8. and conforms to any of the following:
(1) respiratory

Inclusion criteria: 1. Patients aged 18 or older, and meet the diagnostic criteria of Diagnosis and Treatment Scheme of Novel Coronavirus Infected Pneumonia published by the National Health Commission. Criteria for diagnosis (meet all the following criteria):
(1) With epidemiological history;
(2) Clinical manifestations (meet any 2 of the following): fever, normal or decreased white blood cell count or lymphopenia in the early stage of onset, and Chest radiology in the early

Inclusion criteria: 1. Aged >=18 years old;
2. The person, who is diagnosed as COVID-19 according to the 'Diagnosis and treatment of novel coronavirus pneumonia (trial edition 5)', is treated with de-isolation and meets hospital discharge criteria:
Temperature returned to normal over 3 days, respiratory symptoms improved obviously, two consecutive respiratory tract pathogenic nucleic acid detection negative (sampling time interval at least 1 day), can be removed from isolation

Inclusion criteria: (1) Aged 18 to 65 years old;
(2) Patients with mild or normal type who have been diagnosed with new type of coronavirus infection [in accordance with the diagnostic criteria of the Notice on Printing and Distributing New Coronavirus Pneumonia Diagnosis and Treatment Plan (Trial Implementation) issued by the National Health and Medical Commission];
(3) According to the National Health and Health Commission's antiviral treatment for more than 10 days, the COVID-

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Inclusion criteria: 1. Novel coronavirus history before entering the group:
(1) there were travel history or residential history in Wuhan area or other areas with local cases within 14 days before onset;
(2) During the 14 days before onset, there were patients with fever or respiratory symptoms from Wuhan or other areas with persistent local cases;
2. Clinical symptoms and signs:
3. Laboratory examination and chest CT

Inclusion criteria: 1. Aged 18 to 70 years old, inpatients, male or female;
2. Patients with severe COVID-19: confirmed cases shall be in compliance with guideline of "Diagnosis and Treatment Plan for COVID-19 (Version 7)" or updated versions.Confirmed cases can be defined if suspected cases have characteristic of following pathogeny or serology:
1) Detect nucleic acid of novel coronavirus positive by real-time fluorescence RT-PCR;
2) Have highly homologous to known

Inclusion criteria: 1. Patients diagnosed with COVID-19 [in accordance with the diagnostic criteria of the Notice on Printing and Distributing COVID-19 Diagnosis and Treatment Plan (Trial Implementation Version 6th) issued by the National Health and Medical Commission], and meets the discharge criterion;
2. Aged 18 to 75 years old;
3. Be consious;
4. Volunteer to participate in the study and sign the informed consent form, and be able to understand, receive

Inclusion criteria: (1) Aged >=18 years;
(2) 2019-nCoV pneumonia is diagnosed according to the standard of diagnosis and treatment;
(3) Patient's condition has reached the standard of desolation / discharge specified according to the diagnosis and treatment plan;
(4) According to the diagnosis standard, the syndrome differentiation of TCM is FeiPiQiXuZheng;
(5) Volunteer to participate in the study and sign informed consent

Inclusion criteria: 1. Patients are eligible for inclusion if they meet all of the following criteria within 5-7 days after the first admission to the ICU.
2. The patients met the diagnostic criteria of 2019-nCoV pneumonia (new coronavirus infection) pneumonia treatment plan (Fifth version), the National Health Office [2020] No.103.;
3. The primary disease is 2019 nCoV infection;
4. Respiratory distress, RR >=30 times/unit;
5. Under resting state, blood oxygen saturation

Inclusion Criteria:

Male and female, 18-75 years old
Patients previously diagnosed with novel coronavirus pneumonia: the course of illness is no more than 14 days; if the course of the disease was more than 14 days, patient meets one of the following

Inclusion criteria: (1) Meet the diagnostic criteria for patients with COVID-19 of the Proposal of Diagnosis and Treatment of COVID-19;
(2) Aged 18-85 years;
(3) To agree to participate in the study and sign the informed consent

Inclusion criteria: (1) Patients with mild, common, and severe neocoronary pneumonia;
(2) Those who meet the diagnostic criteria for insomnia with traditional Chinese medicine;
(3) Those who meet the diagnostic criteria for insomnia with western medicine;
(4) Those aged 18-75 years;
(5) Agree to receive treatment, and the patient or legal guardian signs the information through mobile phone software and voice authorization Consent;
(6) During this period, the

Inclusion criteria: Positive result in nucleic acid test, medical diagnosis is Corona Virus Disease 2019

Inclusion criteria: (1) Patients aged 18-80 years;
(2) Clear diagnosis of patients with severe COVID-19;
(3) Voluntarily join the trial and sign the informed consent.

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Inclusion criteria: (1) Healthy subjects with negative COVID-19 nucleic acid test and antibody test;
(2) Aged 18-70 years;
(3) Sign the informed consent and volunteer to participate in this study.

Inclusion Criteria:

Willing and able to provide written informed consent prior to performing study procedures
Age ≥18 years, and ≤75 years;

Inclusion criteria: 1. male and female aged 18-70 years;
2. positive result of test for 2019 novel coronavirus nucleic acid;
3. Clinical diagnosed with pneumonia;
4. onset within 7 days (severe cases can be delayed to 14 days);
5. the informed consent shall be signed by me or the authorized agent;
6. subjects who can take oral medication or who receive oral medication;
7. agree to collect clinical samples;
8. female subjects of child-bearing age are not

Inclusion Criteria:

All subjects participating in this clinical study must meet all of the following criteria:

According to the Diagnosis and Treatment Protocol for COVID-19 (Trial Version 5), under the

Inclusion Criteria:

Sign written informed consent;
Age ≥18 years;
Conforms to the NCP Critical and Critical Diagnostic Standards, namely "Pneumonitis Diagnosis and

Inclusion Criteria:

patients with severe COVID-19 pneumonia
willing to give informed consent

Inclusion criteria: Novel coronavirus pneumonia diagnosis and treatment plan (trial version seventh) was revised to meet the diagnostic criteria of the 18-65 year old patients

Inclusion criteria: 1. Mild or moderate cases who meet the "Diagnosis and Treatment Protocol for Novel Coronavirus Pneumonia" issued by the National Health And Health Council are trialled in the fifth and sixth editions of clinical diagnosis and diagnostic criteria;
2. Aged >=18 years;
3. Patient informed consent.

Inclusion Criteria:

Aged 18-65 years;
Voluntarily participate in this clinical trial and sign off "informed consent form";
Diagnosed with severe pneumonia of COVID: respiratory distress, RR >30 times / min; resting

Inclusion criteria: 1) Aged 18 to 70 years old;
2) Infection with new coronavirus pneumonia was confirmed by throat swab nucleic acid test. Patients with severe and critical illness were clinically judged. The symptoms of acute infection have been alleviated after treatment (at least 14 days after onset, no fever for more than 3 days), and the disease is recovering Within three months;
3) CT examination of patients with multiple fibrotic shadows in both lungs;
4) Blood routine, liver, and

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Inclusion criteria: 1. Aged 18 to 75 years old;
2. mild or normal patients;
3. not using chloroquine phosphate, lopinavir other than other antiviral drugs;
4. under the gastroscope, there was mucosal damage and pathological biopsy of 2019-ncov nucleic acid positive;
5. confirmed cases meeting all the following criteria (the 5th edition of the medical guidelines):
(1) epidemiological history;
(2) Clinical manifestations (in accordance with any 2 of the following):

Inclusion Criteria:

COVID-19 has been diagnosed, and the nucleic acid test of respiratory specimens such as sputum or nasopharyngeal swabs has been negative for two consecutive times after treatment (sampling time interval of at least 24 hours);

Inclusion Criteria:

Confirmed COVID-19, and the nucleic acid test of respiratory specimens such as sputum or nasopharyngeal swabs is negative twice after the treatment (sampling interval is at least 24 hours); Negative nucleic acid test of respiratory specimens such as sputum or nasopharyngeal swabs

Inclusion criteria: (1) patients with definite diagnosis of novel coronavirus pneumonia and hospitalized at Leishenshan hospital;
(2) patients aged 18-80 years old;
(3) patients who signed informed consents, voluntarily and persistently participated in the study and follow-up.

Inclusion criteria: 1. Meet the history of COVID-19 confirmed in accordance with the diagnosis and treatment plan of COVID-19(trial seventh edition);
2. Meet the discharge standards of COVID-19(trial seventh edition);
3. Meet the diagnosis standard of qi deficiency and phlegm stasis syndrome;
4. Patients aged 18-70 years old, 28 days after discharge, still have abnormal manifestations on lung CT;
5. Patients willing to sign informed consent

Inclusion Criteria:

Aged between 18 and 60 years.
Able to understand the content of informed consent and willing to sign the informed consent
Able and willing to complete all the secluded study process during the whole 6 months study follow-

Inclusion criteria: 1. Aged 18 years old;
2. diagnosed with viral pneumonia such as new coronavirus;
3. voluntary informed consent

1. Healthy subjects aged 6 years and above;
2. Request medical history and physical examination, and the researchers judged health status of subjects;
3. Since December 2019, subjects have not been to Hubei, overseas and villages/communities with COVID-19 epidemic, and do not contact with COVID-19 cases or suspected cases. Subject shall be

Inclusion criteria: 1. Discharged patients with COVID-19 pneumonia;
2. With dyspnea symptom (mMRC 2~3);
3. Aged 18~75 years old;
4. Have and can use smart phone (study subjects or family members);
5. Obtain the consent of the subjects or legal representative and sign the informed consent.

Inclusion criteria: 1. Patients who have been diagnosed as COVID-19 according to The Diagnosis and Treatment Plan for 2019-nCoV(The Fifth Trial Edition);
2. aged between 18 to 80 years old, male or female;
3. the subjects participating the clinical trial are voluntary and sign the informed consent, thoroughly informed the purpose, method, possible reaction.

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Inclusion Criteria:

Healthy adults aged 18-59 years;
Proven legal identity;
Participants should be capable of understanding the informed consent form, and such form should

Inclusion criteria: (1) Both male and female, 18 years old <= aged <= 75 years old;
(2) The territory is limited to wuhan, hubei province;
(3) Voluntarily participate in the study and sign the informed consent.

(1) novel coronavirus pneumonia has not been stopped since the outbreak of COVID-19;
(2) the co worked personnel were diagnosed as COVID-19 or asymptomatic infection or SARS-CoV-2 IgM positive within 14 days;
(3) the detection of nucleic acid, IgM and IgG of SARS-CoV-2 was negative within 1 weeks;
(4) between 18-70 years old;

Inclusion criteria: 1. Aged >= 18 years old;
2. Patients suspected of having COVID-2019 pneumonia who also have one of the following etiological or serological evidence:
(1) Real-time fluorescent RT-PCR detection of COVID-2019 nucleic acid positive;
(2) viral gene sequencing, and Known novel coronaviruses are highly homologous;
(3) Serum novel coronavirus-specific IgM antibodies and IgG antibodies are positive; serum novel coronavirus-specific IgG antibodies

Inclusion criteria: 1) Aged 18 to 75 years old, male or female;
2) The confirmed participants were diagnosed as mild, severe or critical cases according to the notice on printing and distributing the diagnosis and treatment plan of pneumonia with new coronavirus infection (trial version 7);
3) Those who agree not to participate in other clinical studies within 30 days after administration of the study;
4) Patients voluntarily signed informed consent

Inclusion Criteria:

Clinical diagnosis of the convalescence of COVID-19 (defined by the Chinese Guideline for COVID-19 in 2020 Trial Version 7).
18-70 years old;

Inclusion criteria: 1. Patients who meet the COVID-19 diagnostic criteria and discharge standards with SARS-CoV-2 virus nucleic acid test is negative;
2. The pulmonary imaging in the last 1 week showed inflammation or fibrosis;
3. Aged 18 to 70 years old;
4. Signed informed consent.

Inclusion criteria: 1. Meet the convalescence of COVID-19 diagnostic criteria;
2. Short breath, hyperhidrosis, dry cough, chest distress, palpitation and other clinical discomfort symptoms of lung and heart function decline, in which 3 symptoms appear at the same time, or the visual analogue score of single symptom (VAS) >=4 points;
3. Patients who were discharged for more than 2 weeks were cured;
4. Aged 18-70 years old;
5. Sign informed consent

Inclusion criteria: 1. COVID-19 convalescent diagnostic criteria;
2. Cured and discharged patients more than 2 weeks;
3. Aged from 18 to 70 years;
4. Those with insomnia, anxiety, depression and other sleep mood disorders as the main clinical manifestations, and those with two symptoms at the same time, or visual analogue scale (VAS) of single symptom >=4;
5. Sign the informed consent

Inclusion criteria: 1. The subject or legal agent has signed the informed consent form; agreed to not participate in other clinical researchers within 30 days from the first administration of the study drug to the last administration.
2. Aged 18 to 75 years old;
3. Meet the diagnostic criteria for pneumonia caused by new coronavirus (2019-nCoV) (seventh edition)
4. Treated patients or relapsed patients meet any of the following:
(1) Fever again, or the clinical symptoms

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Inclusion Criteria:

The patient volunteered to participate in the study, approved the aspirin treatment, and was willing to randomly accept one of the aspirin treatment regimens, and provided written informed consent,

Subject is required to meet one of the following criteria for confirmation of a novel coronavirus

Inclusion criteria: (1) Convalescent patients with novel coronavious pneumonia,and the patient is aged 18-75 years old;
(2) The TCM syndrome type conforms to deficiency of 'qi' and 'yin' and not exhausted of 'Yu Xie';
(3) Voluntary test, or the immediate family or the client's approved client agrees to the test, and voluntarily signs the informed consent;
(4) Provide detailed contact information, no short-term willingness to move and be willing to cooperate with followers

Inclusion criteria: 1. COVID-19 confirmed with SARS-COV-2 RNA detection;
2. Hypertension diagnosis;
3. Aged >=18 years.

Inclusion criteria: (1) There is no restrictions on gender and age;
(2) Patients diagnosed with mild pneumonia (slight clinical symptoms, no pneumonia manifestations on imaging), general pneumonia patients (with fever, respiratory tract symptoms, etc., imaging showed pneumonia but no multiple organ damage);
(3) Hospitalized patients;
(4) Volunteer to join the trial and sign the "informed consent;
(5) Promise not to perform other exercise activities

Inclusion criteria: 1. COVID-19 convalescent diagnostic criteria;
2. Cured and discharged patients more than 2 weeks;
3. Aged from 18 to 70 years;
4. Patients with sleep mood disorders such as restlessness, anxiety and sleep disorders were the main clinical manifestations, and those with two symptoms at the same time, or visual analogue scale (VAS) of single symptom >=4;
5. Sign the informed consent

Inclusion criteria: 1. Patient diagnosed with COVID-2019(positive for swab or sputum nucleic acid test);
2. The patients meet the definition of COVID - 19 (seventh edition) in the national health commission of the general and severe type;
3. The patients were in recovery;
4. The age of patients ranged from 18 to 70 years old.

Inclusion criteria: 1. Healthy subjects aged 3 years and above;
2. By asking for a medical history and a physical examination, the researchers determined that the subjects were in good health;
3. From December 2019 to now, the subject has not been to hubei province, outside the country or in a village/community where there has been an outbreak. The subject has not been exposed to a person infected with or suspected of COVID-19;
4. Female subjects with

Inclusion criteria: 1. Healthy subjects with negative covid-19 nucleic acid test and antibody test;
2. Aged 18-70 years old;
3. Signed informed consent and participated in the study voluntarily.

Inclusion criteria: 1. Aged >= 18 years, male or female.
2. Patients who were diagnosed as COVID-19 according to the The Diagnosis and Treatment Scheme of COVID-19(The 7th Trial Edition).
3. The condition meets the discharge standards stipulated in the The Diagnosis and Treatment Scheme of COVID-19(The 7th Trial Edition) after treatment.
4. The syndrome differentiations are the deficiency of lung and spleen qi at the time of screening. According to the

Inclusion criteria: (1) Patients who meet the COVID-19 diagnostic criteria and discharge standards, but the later SARS-CoV-2 virus nucleic acid test is positive;
(2) Aged 18 to 70 years old;
(3) Signed informed consent.

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Inclusion criteria: 1. COVID-19 patients accompanied with gastrointestinal symptoms;
2. COVID-19 patients accompanied with antibiotic-associated diarrhea

Inclusion criteria: (1) age =18 years old, gender not limited;
(2) Laboratory (RT-PCR) confirmed COVID-19;
(3) the time from the first positive nucleic acid test to randomization does not exceed more than 4 days;
(4) informed consent has been signed.

Inclusion Criteria:

Healthy adults aged ≥60 years;
Be able to understand and sign the informed consent voluntarily;
Provide legal identification;

Inclusion Criteria:

Patients confirmed with COVID-19 and classified as severe

Inclusion criteria: 1. Aged 2 to 65 years old, and signed informed consent;
2. According to the diagnostic criteria of the "New Coronavirus Pneumonia Diagnosis and Treatment Program (Trial Version 7)" issued by the National Health and Health Commission, those who have been diagnosed with CoVID-19 of light and common type and have been treated with hydroxychloroquine and Abidol but nCoV Nucleic acid did not turn negative. The specific diagnostic criteria are as follows:
(1)

Inclusion criteria: 1. Meet the diagnostic criteria of the n Novel Coronavirus Pneumonia (COVID-19) in the recovery period;
2. Patients with fatigue, anorexia, abdominal distension, loose stools and other digestive dysfunction as the main clinical manifestations, 3 of them at the same time, or a single symptom visual analog score (VAS) greater than 5 points;
3. Heal patients who have been discharged for more than 2 weeks;
4. Aged 18-70 years old;
5. Sign the informed

Inclusion criteria: 1. According to the standard or the Chinese version New Coronavirus Pneumonia Diagnosis and Treatment Program (5th edition), the participant is clearly diagnosed as Coronavirus Disease 2019 (COVID-19) and classified as mild or common type;
2. During treatment in hospital, the condition tends to be stable;
3. Write the informed consent form and be able to insist on long-term follow-up

Inclusion criteria: 1. Patients with confirmed new coronavirus-infected pneumonia;
2. Aged >=18 years old;
3. Voluntarily sign written informed consent.

Inclusion Criteria:

Phase Ia:

Healthy adults aged 18 to 59 years (including boundary values), both men and women

Inclusion criteria: 1. Patients have been confirmed diagnosis from a positive test for COVID-19 RT-PCR,>18 years old;
2. Patients who were classified as mild or common clinical types according to the Diagnosis and Treatment Protocol for Novel Coronavirus Infection-Induced Pneumonia version 5 (trial);
3. Patients who were able to skillfully use the mobile phone app.

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Inclusion criteria: 1. The novel coronavirus pneumonia, once diagnosed as a severe or critical illness, has been cured and discharged without any pathogen or rehabilitation;
2. It conforms to the above diagnostic standards of Western medicine and TCM syndrome differentiation;
3. The lung function indexes meet the following standards:
(1) the percentage of forced vital capacity in predicted value (FVC%) is between 50% - 90%;
(2) the percentage of carbon monoxide

Inclusion criteria: 1. Adults (defined as those aged >=18 years) ;
2. The CT findings of patients with novel coronavirus infection or pneumonia confirmed by PCR;
3. In-patients, accompanied by fever (Temperature at Armpit > 37 degree C) or cough, dyspnea;
4. The interval between symptom onset and randomization was less than 8 days;
5. Agree to participate in the study and accept the randomization;
6. Who has not participated in a drug clinical trial within 3

Inclusion criteria: (1) There is no restrictions on gender and age;
(2) Patients diagnosed with mild pneumonia (slight clinical symptoms, no pneumonia manifestations on imaging), general pneumonia patients (with fever, respiratory tract symptoms, etc., imaging showed pneumonia but no multiple organ damage);
(3) Hospitalized patients;
(4) Volunteer to join the trial and sign the "informed consent;
(5) Promise not to perform other exercise activities

Inclusion criteria: 1. Conform to the diagnostic criteria of Western Medicine's New Coronary Pneumonia;
2. Aged 18-65 years old male or female.

Inclusion Criteria:

Male and female subjects aged 18 to 45 years, inclusive;
The body weight no less than 50 kg for male subjects and no less than 45 kg for female subjects.
Body mass index (BMI) = weight (kg)/square of height (m2), ranging from 18-28 kg/m2 (including the

Inclusion criteria: 1. Aged between 2 and 18 years male or female;
2. Laboratory (RT-PCR) and clinical symptoms confirmed case of COVID-19, according to the official guideline "Pneumonia Diagnosis and Treatment Scheme for Novel Coronavirus Infection (Trial Version 6) ". Or the patients with negative 2019-nCoV RNA results but clinical symptoms suspected;
3. The legal guardian of the enrolled children shall sign the informed consent voluntarily (subjects >=8 years old shall sign

Inclusion Criteria:

Comply with the observation age of this clinical trial: 18-59 years old (both included) adults.
The subjects themselves voluntarily agreed to participate in the study, and signed an informed consent form, and can provide legal identification; understand and abide by the requirements of the

Inclusion criteria:
 1. Patients with COVID-19 who had mild symptoms in line with the diagnostic criteria of the Chinese Management Guidelines for COVID-19 (version 6.0)
2. Good communication and understanding of Chinese
 3. Aged 20-69 years

Inclusion criteria: 1. Healthy adults aged 18-59 years, or 60 years and above who can provide legal identity certificate of their own, both males and females are acceptable;
2. Subjects who understand the contents of the informed consent form and the background of the investigational vaccine, agree to sign the informed consent form voluntarily, and are able to use thermometer and scale, and may fill in the diary card and contact card as required;
3. Subjects who are able to

Inclusion Criteria:

Persons with full civil capacity aged 18-59 years (both included);
The subjects themselves voluntarily agreed to participate in the study, and signed an informed consent form, and can provide valid identification; understand and comply with the requirements of

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Phase Ib:

Inclusion Criteria:

Subject must be 18 to 49 years of age inclusive;
Body weight ≤100 kg and body mass index (BMI) within the range of 19.0-24.0kg/m2 (inclusive);
Male or female;
Inclusion Criteria:

Subject must be 18 to 49 years of age inclusive;
Body weight ≤100 kg and body mass index (BMI) within the range of 19.0-24.0kg/m2 (inclusive);
Male or female;
Inclusion Criteria:

Able to provide written informed consent
Males or females. Aged ≥ 18 years old
Body mass index (BMI) between 18.0 and 26.0 kg/m2
Inclusion criteria: 1. Patients who met the discharge standards with the criteria for Diagnosis and Treatment Guideline of Novel Coronavirus Pneumonia (Trial 7th edition) and have a negative nucleic acid test for the new coronavirus after discharge
2. lung CT showed inflammation in the lungs or manifestations of pulmonary fibrosis in the past 1 week;
3. Aged 18-70 years old;
4. Sign informed consent
Inclusion criteria: 1) The subject fully understands the purpose, nature, method and possible adverse reactions of the study, volunteers to be a subject, and signs the informed consent prior to the commencement of any study procedure, and warrants that he/she will participate in any study procedure;

2) The subject can have good communication with the researcher, and understand and comply standard constrain ;

Persons with full capacity for civil conduct aged ≥60 years;
Subjects voluntarily agree to participate in the research and sign an informed consent form, and can provide valid identification; understand and comply with the requirements of the trial protocol; have
Inclusion Criteria:

For adult group (age ≥18 and ≤55)

Male or female subjects of ≥18 years old and ≤55 years old with body mass index (BMI) ≥18 and
Inclusion Criteria:

Subjects are fully aware of the purpose, nature, methods and possible adverse reactions of the study, voluntarily serve as subjects, and sign the informed consent form before the start of any study procedures, and ensure that he/she will participate in any procedures in the study in person;
Inclusion Criteria:

Male or female subjects of ≥ 18 years old with body mass index (BMI) ≥ 18.5 and ≤ 30 at the Screening Visit.
The subject can provide with informed consent and signs and dates a written informed consent form

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Inclusion criteria: 1. Subjects over 18 years old;
2. Patients with novel coronavirus pneumonia history;
3. Those who can read Chinese characters;
4. Those who can independently cooperate with doctors to complete various scales and corresponding tests;
5. Subjects with mobile communication equipment such as mobile phones or tablets;
6. Mobile equipment can be connected to the Internet at any time;
7. The patient himself had informed consent in the

Inclusion criteria: 1) Aged >=18 years old at the time of enrollment;
2) Be able to comply with the requirements of the clinical research protocol, complete all trial procedures, and sign the informed consent form;
3) The nucleic acid test of SARS-CoV-2 is negative;
4) The serum total antibody test of SARS-CoV-2 is negative;
5) The lung CT imaging is normal (no imaging features of COVID-19);
6) The HIV screening result is negative;
7) For female subjects,

Inclusion criteria: 1. Male or female volunteers, age: 18-59 years old (including threshold).
2. Body weight: male >= 50 kg, female >= 45 kg; body mass index (BMI) in the range of 18-30 kg / m2 (including critical value).
3. From signing the informed consent form to 12 months after the last vaccine injection, the fertile female subjects or partners with fertile male subjects agreed and could take effective contraceptive measures, such as avoiding sexual life or using oral contraceptives

Inclusion Criteria:

Healthy children and adolescents aged 3-17 years;
The subject and/or guardian can understand and voluntarily sign the informed consent form (double sign required for 8-17 years old);

Inclusion criteria: 1. Patients who meet the high risk assessment criteria for avascular necrosis of the femoral head after recovery of patients with new coronavirus pneumonia;
2. Patients aged 18-70 years old;
3. Patients who voluntarily participate in clinical research and sign informed consent.

Inclusion criteria: 1. Meet the high risk assessment criteria of avascular necrosis of femoral head in patients with new coronavirus pneumonia after recovery;
2. Patients aged 18-70 years old;
3. Patients who voluntarily participate in clinical research and sign informed consent.

Inclusion Criteria:

Aged 18 to 55 years old in MID group; aged 6-17 years old in MIN group; aged 56 years and above in OLD group; aged in EBO group is not limited.
Able to understand the content of informed consent and willing to sign the informed consent

Inclusion Criteria:

Age of 18-65 years (inclusive) ,women or man
SARS-CoV-2 detected in the diagnostic specimen (nasopharyngeal swab)
High homology of viral gene sequencing with the known SARS-CoV-2 "

Inclusion criteria: 1. Patients who meet the recovery period of new coronavirus pneumonia;
2. Syndrome differentiation is of Qi and Yin deficiency type;
3. Informed consent to this study, voluntarily participate in clinical research;
4. Can provide detailed contact information, have no short-term relocation wishes, and are willing to cooperate with follow-up.

1. Healthy permanent residents aged 18 years and above;
2. Subjects agree to sign the informed consent forms voluntarily;
3. Subjects are able to comply with the requirements of the clinical trial protocol;
4. Armpit temperature <=37.0 degree C;
5. Female subjects of childbearing age were not pregnant at the time of enrollment, were not

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Inclusion Criteria:

1) Male or female, aged between 18 and 75 (including boundary values) at screening.

2) Severe or critically ill patients who have been diagnosed with a novel coronavirus during

Inclusion criteria: 1. Healthy adults aged 18-59 who can provide the legal identification of the subjects, both male and female;
2. The subjects knew the content of the informed consent and the vaccination situation, voluntarily signed the informed consent, and had the ability to use the thermometer, scale and fill in the diary card and contact card as required;
3. Those who can communicate well with researchers, and understand and comply with the requirements of this study;

Inclusion criteria: The inclusion criteria of study participants included:
1. According to the diagnosis and clinical management of New Coronavirus infection in 2019 (trial version 6 or trial version 7), COVID-19 was diagnosed;
2. Patients with pcl-c score >= 50;
3. Patients willing to participate in the study.

Inclusion criteria: 1. Healthy permanent residents aged 18 years and above;
2. Subjects agree to sign the informed consent forms voluntarily;
3. Subjects are able to comply with the requirements of the clinical trial protocol;
4. Armpit temperature <= 37.0 degrees C;
5. Female subjects of childbearing age were not pregnant at the time of enrollment, were not breastfeeding, and had no birth plan within the first 3 months after enrollment; effective

Inclusion Criteria:

Healthy adults aged 18-59 or elderly aged ≥60 years;
Proven legal identity.

Inclusion criteria: 1. Subjects over 18 years old at the time of enrollment;
2. The subjects who can comply with the requirements of the clinical study protocol, complete all the trial procedures, and sign the informed consent form;
3. Since December 2019, the subjects who have not been to the epidemic area, have not contacted the new coronal infection or suspected cases, and are in the non isolation period, and there are no new coronal infection or suspected cases in the same village /

Inclusion Criteria:

Patients diagnosed with mild or moderate COVID-19 (as per the Diagnosis and Treatment Protocol for Novel Coronavirus Pneumonia (Trial, 8th edition), China, or Clinical Management of COVID-19, WHO).

Inclusion Criteria:

Male or female participants between the ages of 18 and 85 years, inclusive, at randomization.
Participants who are willing and able to comply with all scheduled visits, vaccination plan, laboratory tests, lifestyle considerations, and other study procedures.

Inclusion Criteria:

Healthy subjects of ≥ 18 years old.
The subject can understand and voluntarily sign the informed consent.
The subject can The subject canprovide legal identification.

Inclusion Criteria:

Aged 18 years and above.
Able to understand the content of informed consent and willing to sign the informed consent.
Able and willing to complete all the secluded study process during the whole study follow-up period.

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Inclusion criteria:
1.Age >= 18 years old and < 60 years old, regardless of gender (for adult subjects only);
2.Age >= 60 years old and <= 85 years old, regardless of gender (for elderly subjects);
3.Body mass index (BMI) is between 18-35 kg/m2 (including upper and lower limits);
4.Axillary temperature < 37.3 degrees C on the day of vaccination

Inclusion criteria: 1. People who are 18 years old and above;
2. The subjects themselves voluntarily participate in the study, sign an informed consent form, and can provide valid identification, understand and comply with the requirements of the trial protocol;
3. Female subjects of childbearing age agree to take effective contraceptive measures from the beginning of the study to 2 months after the full vaccination

Inclusion Criteria:

Patients with mild and severe COVID-19 confirmed according to the Diagnosis and Treatment Protocol for Novel Coronavirus Pneumonia (Trial Version 7) that was issued by the National Health Commission & State Administration of Traditional Chinese Medicine;

Inclusion criteria: 1. Aged from 18 to 70 years old;
2. The diagnostic criteria of subjects clearly defined as "SARS-CoV-2 infected pneumonia" shall be based on the criteria of severe patients stipulated in the "Novel Coronavirus Infected Pneumonia Protocol (Trial version vii)" issued by the National Health Commission;
3. Patients can receive aerosol inhalation;
4. Consent to sign a written informed statement prior to the beginning of the study

Inclusion Criteria:

Subject must be 18 to 49 years of age inclusive
Body weight ≤100 kg and body mass index (BMI) within the range of 19.0-26.0kg/m2 (inclusive).
Male or female

Inclusion criteria: 1. Subjects aged 18-59 years;
2. The subjects who signed the informed consent form and participated voluntarily;
3. Subjects with good compliance;
4. Subjects with the interval of more than 14 days from the last vaccination;
5. Before entering this study, subjects with history and clinical examination and body temperature <= 37.0 degrees C.

Inclusion criteria: 1. Subjects aged 18-59 years old;
2. The subjects who signed the informed consent form and participated voluntarily;
3. Subjects with good compliance;
4. Subjects with the interval of more than 14 days from the last vaccination;
5. Before entering this study, subjects with history and clinical examination and body temperature <= 37.0 degrees C.

Inclusion criteria: 1. Subjects aged 18-59 years;
2. The subjects who signed the informed consent form and participated voluntarily;
3. Subjects with good compliance;
4. Subjects with the interval of more than 14 days from the last vaccination;
5. Before entering this study, subjects with history and clinical examination and body temperature <= 37.0 degrees C.

Inclusion criteria: 1. A healthy person aged 18-59 who can provide the legal identification of the subjects, both male and female;
2. The subjects who knew the content of informed consent and the vaccination situation voluntarily signed the informed consent, and had the ability to use the thermometer, scale and fill in the diary card and contact card as required;
3. Subjects who can communicate well with researchers and understand and comply with the requirements of this study;

1. Patients with confirmed COVID-19 by Pathogen testing;
2. Above 18 years old (inclusive), both male and female;
3. Severe novel coronavirus pneumonia meets one of the following conditions;
(1) Shortness of breath, RR>=30 times/min;
(2) In the resting state, the oxygen saturation is less than or equal to 93% when inhaling air;

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1. Patients with confirmed COVID-19 by Pathogen testing;
2. Above 18 years old (inclusive);
3. At the time of enrollment, the patient had any of the main symptoms (fever, cough, expectoration, chest tightness, polypnea, dyspnea);
4. Voluntarily sign written informed consent

1. Asymptomatically infected with novel coronavirus pneumonia discovered through pathogenic testing;
2. Over 18 years old (including 18 years old);
3. Voluntarily sign a written informed consent before the study begins.

Inclusion criteria: 1. Male and female subjects aged 18 to 45 years, inclusive;
2. The body weight no less than 50 kg for male subjects and no less than 45 kg for female subjects. Body mass index (BMI) = weight (kg)/square of height (m2), ranging from 18-28 kg/m2 (including the critical value);
3. These indicators are normal including physical examination, vital signs, laboratory tests and other accessory examinations (chest radiology, abdominal B-mode ultrasonography, ECG

Inclusion Criteria:

Aged 18-85 years old
Able to understand the content of informed consent and willing to sign the informed consent.
Able and willing to complete all the secluded study process during the whole study follow-up period

1) Patients who met the diagnostic criteria of covid-19 and discharged after treatment;
2) Aged 18-75 years;
3) Having one or more clinical manifestations of respiratory symptoms or fatigue;
4) Patient informed consent.

Inclusion criteria: 1. Confirmed as a close contact with the novel coronavirus pneumonia through flow investigation;
2. Over 12 years old (including 12 years old), no gender limit;
3. Voluntarily sign a written informed consent before the start of the study.

Inclusion criteria: 1. Meet the diagnostic criteria for the recovery period of new coronavirus pneumonia;
2. Aged from 18 to 70 years old;
3. Those who have depression, anxiety, poor sleep, fatigue and other sleep mood disorders as their main clinical manifestations, and those who have 2 symptoms at the same time;
4. Screening and baseline Hamilton Depression Scale (HAMD) 17 points >=17 points, <=24 points;
5. Patient Health Questionnaire PHQ-15>=5

Inclusion Criteria:

Subject ≥ 18 years, signing the informed consent.
SARS-CoV-2 infection by PCR ≤ 7 days
One or more of COVID-19 related symptoms or measured fever present within 48 hours prior to

1. Request medical history and physical examination, and the researchers judged health status of subjects;
2. Aged 18 to 60 years;
3. BMI 18.5-30Kg/m2;
4. Novel Coronavirus (COVID-19) antibodies (IgG, IgM) are negative;

Inclusion Criteria:

Healthy adults aged 18-59 years;
The subject can understand and voluntarily sign the informed consent form;
Proven legal identity

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Inclusion Criteria:

Healthy subjects of ≥ 18 years old.
The subject can understand and voluntarily sign the informed consent.
Axillary temperature <37.0℃.
For the diagnosis of light and common COVID-19, refer to COVID-19 Diagnosis and Treatment Protocol (Trial Eighth Edition) (hereinafter referred to as "Diagnosis and Treatment Protocol");
Reference for asymptomatic infected persons: COVID-19 Control Plan (7th Edition).
1. Conform to the diagnostic criteria of mild, common COVID-19 and novel Coronavirus asymptomatic infection in the two protocols mentioned above;

Inclusion Criteria:

The subjects ≥ 18 years old who has completed one dose of recombinant Ad5 vectored COVID-19 vaccine.
The subjects can provide with informed consent and sign informed consent form (ICF).

Subjects who comply with the following criteria will be enrolled:
1. Willing to participate in the study with informed consent;
2. Health male or female adults (i.e. aged 18 years and above);
3. Without a history of traveling or residence in domestic areas of high and moderate pandemic risk, overseas or epidemic areas, nor a history of contact with confirmed, asymptomatic or suspected

Subjects who comply with the following criteria will be enrolled:
1. Willing to participate in the study with informed consent;
2. Health male or female adults (i.e. aged 18 years and above);
3. BMI within 18-28 kg/m2 (including both boundaries);
4. Without a history of traveling or residence in domestic areas of high and moderate pandemic risk

Aged 18-59 years.

Inclusion Criteria:

Healthy adults 18 years of age and above at the time of enrollment;
Able to provide consent to participate in and having signed an Informed Consent Form (ICF);
Able and willing to complete all the scheduled study procedures during the whole study follow-up

Inclusion Criteria:

Age range: healthy people aged 3 years and and older who can provide legal identification;
The subject and/or his legal guardian and/or his entrusted person can understand the study procedures and informed consent, voluntarily sign informed consent form, and be able to comply

Inclusion Criteria:

Aged 3 years and above (after enrolled, subjects will be allocated according to age).
By asking for medical history and physical examination, the health condition judged by the investigators is well
1. Chinese aged 18 or above who can provide legal proof of identity;
2. According to the medical history and physical examination, the researcher determined that the patient was healthy, both male and female;
3. Having the ability to understand the study procedures, have informed consent, voluntarily signed informed consent, can comply with the requirements of the clinical study protocol, having the ability

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Inclusion Criteria:

Healthy children and adolescents aged 3-17 years;
The subjects and/or guardians can understand and voluntarily sign the informed consent form (For subjects aged 8-17 years, both subjects and guardians need to sign the informed consent form);

1. Aged >=18 years old when enrolled;
2. Participants signing the informed consent;
3. Being able to participate in all planned follow-ups and accomplish all research procedures including completing a diary card and coming back for interview;
4. Having not been administered any pneumonia vaccine or COVID-19 vaccine, and having not

1. Aged >=18 years when enrolled;
2. Participants signing the informed consent;
3. Being able to participate in all planned follow-ups and accomplish all research procedures including completing a diary card and coming back for interview;
4. Having not been administered any pneumonia vaccine or COVID-19 vaccine, and having not

1) Conform to the diagnostic criteria of COVID-19 (Guidelines for the diagnosis and treatment of coronavirus disease 2020(trial version eighth)) Patients who met the discharge standard and had negative neocoronavirus nucleic acid detection after discharge
2) Aged 18-75 years;
3) Having clinical manifestations of respiratory symptoms or fatigue;

Inclusion Criteria:

the patients diagnosed as the common or severe cases of COVID-19
aged 18-85
the absolute lymphocyte counts below the normal value (≤1.1-3.2×10⁹/L)

Inclusion Criteria:

6-59 years of age at the time of enrollment;
Able to provide consent or obtain consent from guardian to participate in the trial and sign an Informed Consent Form (ICF);

Inclusion Criteria:

Health subjects aged 18-59 years, who have been completed two-dose regimen of inactive SARS-CoV-2 vaccine in the past 3-6 months, or received one dose of inactive SARS-CoV-2 vaccine in the past 1-3 months

1) Be at least 18 years old at the time of entry;
2) Able to comply with the requirements of the clinical research plan to complete all trial procedures and sign the informed consent form;
3) For female subjects: infertility (surgical sterilization or at least one year after menopause) or willing to use appropriate contraceptive measures from 30 days before vaccination to 3 months after

1. Aged over 18 years;
2. Received two doses of the new crown inactivated vaccine at least 4 months ago;
3. According to the medical history and physical examination, the researchers judged to be healthy (excluding the following conditions: known or suspected to suffer from more serious diseases at the same time, including: respiratory diseases, acute infections or active chronic diseases, liver and

1. Meet the diagnostic criteria for COVID-19 (National Eighth Edition);
2. Aged over 18 years;
3. Normal body temperature within 48 hours;
4. In the case of inhaling air, SpO2>93%;
5. The onset time is more than 7 days;

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Inclusion Criteria:

Male or female ≥18 years of age.
Individuals are willing and able to comply with study requirements, including all scheduled visits, vaccinations, laboratory tests and other study procedures

Inclusion Criteria:

A signed informed consent form (ICF) from the patient or their legally authorized representative.

Inclusion Criteria:

Healthy males or females, ages ≥18 to ≤45 years. Male subjects with weight ≥50 kg, or female

Inclusion Criteria:

Healthy population aged 18 years and above;
The subjects can understand and voluntarily sign the informed consent form;
Proven legal identity

Inclusion Criteria:

Aged 3-17 years old (all inclusive);
The subject voluntarily agrees to participate in the study (or the legal guardian of the subject voluntarily agrees the child to participate in the study), and the guardian and the subject (8-17 years

Inclusion Criteria:

Healthy individuals aged 18-59 years as well as 60 years and above who can provide legal identification (males and females are both required).
Willing to participate in the study with informed consent prior to screening

Inclusion Criteria:

Healthy individuals aged 18-59 years as well as 60 years and above who can provide legal identification (males and females are both required).
Willing to participate in the study with informed consent prior to screening

Inclusion Criteria:

Healthy adults aged 18 years and above;
Participants should be capable of understanding the informed consent form, and such form should be signed prior to enrolment ;

Inclusion Criteria:

Healthy residents aged 3 to 17 years at the time of consent
Subjects and/or their guardian agree to sign the informed consent forms voluntarily.
Be able to comply with study requirements/procedures

Inclusion Criteria:

Healthy residents aged 3 to 17 years at the time of consent
Subjects and/or their guardian agree to sign the informed consent forms voluntarily.
Be able to comply with study requirements/procedures

Inclusion Criteria:

Healthy adults aged 26-45;
The subjects can understand and voluntarily sign the informed consent form ;
Proven legal identity

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Inclusion Criteria:

Aged 6-17 years ;
The subject and/or guardian can understand and voluntarily sign the informed consent form (double signature is required for 8-17 years old) ;

1. People aged 18 and above;
2. The subject has independent judgment, can read, understand and complete the vaccination diary card, and voluntarily participate in the vaccination and sign the informed consent form;
3. The subjects can comply with the requirements of the clinical trial protocol;
4. Armpit body temperature <=37.2 degree C;

(1) Diagnostic criteria of discharged patients who met the COVID-19 Diagnosis and Treatment Protocol (Trial Version 6 and Version 7);
(2) Patients who meet the TCM diagnostic criteria of deficiency of lung and spleen and deficiency of Qi and Yin;
(3) Aged between 18 and 65 years;

(1) Diagnosis of light or common type of delta novel coronavirus pneumonia (COVID-19) .The onset was within 7 days.
(2) Informed consent, voluntary test; The process of obtaining informed consent shall be in accordance of GCP.

(1) Meets the diagnostic criteria for COVID-19 (National Edition 8);
(2) Over 18 years old;
(3) Normal temperature within 48 hours;
(4) With air intake, SpO2>93%;
(5) Onset time >=7 days;

(1) COVID-19 close contacts at centralized isolation sites;
(2) Aged >= 18 years;
(3) Agree to participate in the trial, and the patient or the legal guardian signed the informed consent through paper signature, electronic signature of mobile phone software, or voice authorization.

1. Healthy subjects aged 18-59 years as well as 60 years and above who are capable of providing legal identities. Both males and females should be included;
2. The subjects shall fully understand the content of the informed consent form and the study procedure of this clinical trial. The subjects shall voluntarily sign the informed consent form. The subjects shall be capable of using the thermometer and scale, as well as filling in the Diary.

1. Healthy subjects aged 60 years and above who were capable of providing legal identities. Both males and females should be included;
2. The subjects shall fully understand the content of the informed consent form and the study procedure of this clinical trial. The subjects shall voluntarily sign the informed consent form. The subjects shall be capable of using the thermometer and scale, as well as filling in the Diary.

1. Not yet vaccinated prior to the Screening.
2. Diagnosed with any forms of uveitis in any eye.
3. An inactive disease status (all the following criteria are met):
(1)Anterior chamber cells of 0.5+ or less;
(2)Vitreous haze of 0.5+ or less;

1. Age at the time of the first dose of vaccination: the adult group is 18-59 years old (including the boundary value), the elderly group >=60 years old;
2. Those whose axillary body temperature is less than 37.3 degree C on the day of enrollment;
3. Based on the medical history and relevant physical examination and laboratory examination results, the investigator clinically judges those who are healthy;

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All subjects are required to meet all the following inclusion criteria:
1. Healthy adults aged 18 years and above. Both males and females should be included;
2. The subjects shall fully understand the content of the informed consent form and the study procedure of this clinical trial. The subjects shall be voluntary and capable of signing the informed consent form;

1. Discharged COVID-19 patients;
2. Aged from 18 to 75 years;
3. Those who have fatigue symptom;
4. Have a smart phone and can use it independently or with the help of family members;
5. Volunteer to participate in this study and sign an informed consent form

Inclusion Criteria:

aged 18 to 65 years old (including 18 and 65 years);
male and non-pregnant female;
respiratory tract specimens or hematology samples detected positive results of SARS-CoV-2 by real-

Inclusion Criteria:

At the time of signing ICF, the subjects were 18 to 65 years old (including 18 and 65 years old), both men and women;
Within 72 hours before screening, any samples confirmed by the laboratory were positive for SARS-

(1) Meet NCP critical diagnosis criteria;
(2) Volunteer to participate in this study, cooperate with examination and treatment;
(3) The selected patients were basically stable, conscious and cooperative;
(4) No serious mental disorder;
(5) Aged between 20-80 years

Inclusion Criteria:

Health subjects aged ≥18 years, completed two dose of inactive SARS-CoV-2 vaccine in the past 3-9 months.
The subject can provide with informed consent and sign informed consent form (ICF)

1. Discharged COVID-19 patients;
2. Aged from 18 to 75 years;
3. Those who have fatigue symptom;
4. Have a smart phone and can use it independently or with the help of family members;
5. Volunteer to participate in this study and sign an informed consent form

Inclusion Criteria:

Health subjects aged 60 and above, who have been completed two-dose regimen of inactive SARS-CoV-2 vaccine in the past 3-6 months, or received one dose of inactive SARS-CoV-2 vaccine in the past 1-3 months

Inclusion Criteria:

Healthy adults aged 18-59;
Have been received two doses of inactivated SARS-CoV-2 vaccine(CoronaVac)manufactured by Sinovac Research & Development Co., Ltd at an interval of 21 to 31 days for group A Have been

Inclusion Criteria:

≥18 years old when enrolled;
Participants signing the informed consent;
Being able to participate in all planned follow-ups and accomplish all research procedures including

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Inclusion Criteria:

Severe acute respiratory syndrome coronavirus2 (SARS-CoV-2) infection
Within 7 days from the onset of clinical symptoms or 4 days from the onset of severe symptoms
Consistent with the National Health Committee New Coronavirus pneumonia diagnosis and

Inclusion Criteria:

Adults aged 18-59;
Proven legal identity;
The subjects can understand and voluntarily sign the informed consent form and be willing to

Inclusion Criteria:

Persons aged 18-59 with full capacity for civil conduct;
Subjects voluntarily participate in the study, sign informed consent, provide valid identification, understand and comply with the requirements of the study protocol;

The inclusion criteria of rehabilitation training intervention subjects were: 1. COVID-19 patients; 2. Discharged from hospital; 3. Respiratory dysfunction or motor dysfunction or self-care ability disorder according to relevant functional assessment; 4. Aged 18-70 years; 5. Willing to accept rehabilitation training of this program.

1. Male and female, aged 18-70 years; 2. Novel coronavirus nucleic acid test positive; 3. The diagnostic criteria for confirmed cases in the national COVID-19 Protocol (Eighth edition) were met; 4. Onset within 7 days (severe cases can be extended to less than 10 days); 5. The informed consent is signed by patients or their authorized agent; 6. Subjects who may or receive oral medication; 7. Agree to collect clinical samples; 8. Female subjects of reproductive age were not

1) Persons aged 18 and above; 2) The subject has independent judgment ability, can read, understand and complete the vaccination diary card, and voluntarily participates in it and signs the informed consent; 3) The subject can comply with the requirements of the clinical trial protocol; 4) Armpit temperature <= 37.2 degrees C; 5) No birth plan in the first 3 months after enrollment (including male subjects' partners); Effective contraceptive measures were taken within 2 weeks

1. Patients diagnosed with COVID-19 diagnosed in accordance with the "COVID-19 Diagnosis and Treatment Plan (Trial Eighth Revised Edition)" formulated by the National Health Commission of the People's Republic of China or those who are asymptomatically infected with COVID-19 diagnosed in accordance with the "COVID-19 Prevention and Control Plan (Eighth Edition)";
2. The age of the subject at the time of signing the informed consent form is 18 to 65 years old

1.Aged 18-60 years(including cut-off value), gender not limited;
2.Sputum/nasal swabs/pharyngeal swabs/lower respiratory tract secretions and other specimens, RT-PCR detected the new coronavirus nucleic acid positive, or the viral gene sequencing was highly homologous with the known Delta virus;
3.Patients diagnosed with novel coronavirus meet the diagnostic criteria of the pneumonia diagnosis

All inclusion criteria must be met to qualify for inclusion:
1. The age should be over 18 years old and under 70 years old, with no gender limit. Those who meet the diagnostic criteria of clinical pneumonia
2. According to the clinical diagnosis of viral pneumonia:
(1) Fever (oral temperature >= 38 degrees C, or axillary temperature >= 37.5 degrees C; or fever

Inclusion Criteria:

Healthy subjects aged 18 and above with full capacity for civil conduct, who can provide valid identification;
The subjects voluntarily agree to participate in the study and sign the informed consent to

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1.Individuals aged 18-59 years and are able to provide legal identity certificate. Both male and female should be included.
2.The subjects shall fully understand the content of the informed consent form and the characteristics of the study vaccine, voluntarily sign the informed consent form, as well as have the ability to use thermometer, scale and fill in diary card and contact card as required

Inclusion Criteria:

Children aged 3-5 years;
The subject and/or guardian can understand and voluntarily sign the informed consent form;
Proven legal identity

The subjects eligible for this study must meet all of the following inclusion criteria:
1. Aged >= 18 years, male or female.
2. Body mass index (BMI) of 18-35 kg/m2 (inclusive).
3. The subjects in Cohort A have received two doses of the COVID-19 inactivated vaccine (approved) within 3 months (+15 days) and have been dosed at a time interval of 21 (+10) days

1. Aged 18 to 59 years;
2. Refer to the "New Coronavirus Pneumonia Diagnosis and Treatment Plan (Trial Eighth Edition)" formulated by the National Health Commission, with evidence of etiology or serology, and confirmed mild patients with new coronavirus pneumonia (mild clinical symptoms, no imaging Pneumonia manifestations)

Inclusion Criteria:

Healthy children aged 3-17;
The subjects and/or guardians can understand and voluntarily sign the informed consent form (For subjects aged 8-17 years, both subjects and guardians need to sign the informed consent form)

Inclusion criteria for pulmonary tuberculosis patients:

Pulmonary tuberculosis patients aged 18-75 years who have not received COVID-19 vaccine.
The condition is determined to be stable by the clinician.
The subjects can provide with informed consent and sign informed consent form (ICF)

Inclusion Criteria:

Adults aged 18-45;
Proven legal identity;
The subjects can understand and voluntarily sign the informed consent form and be willing to

Inclusion Criteria:

Healthy participants aged 18 years and above;
Proven legal identity;
The subjects can understand and voluntarily sign the informed consent form and be willing to

Main Inclusion Criteria:

First onset of COVID-19 symptoms <7 days at randomization, symptoms such as fever and/or chills, fatigue, muscle or body aches, headache, new loss of taste or smell, sore throat, congestion or runny nose, nausea or vomiting, and diarrhea

1. Healthy individuals aged 18-59 years as well as 60 years and above who can provide legal identification (males and females are both required).
2. Individuals who are able to understand the contents in the informed consent form and information about the investigational vaccine used in this study; willing to sign the informed consent form; able to use the thermometer, scale as well as fill in the diary card and contact card as required

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Inclusion Criteria:

Male or female healthy subjects
Body weight ≥ 50 kg for males and ≥ 45 kg for females
Female subjects must meet the following criteria: not of childbearing potential (e.g., documented

Inclusion Criteria:

18 ≤age ≤59 years;
Vaccinated with two doses of SARS-CoV-2 vaccine (manufactured by Sino Biopharmaceutical Limited or SINOVAC BIOTECH CO., LTD.) (≥6 months from the time of vaccination)

Inclusion Criteria:

18 ≤age ≤59 years;
Vaccinated with two doses of SARS-CoV-2 vaccine (manufactured by Sino Biopharmaceutical Limited or SINOVAC BIOTECH CO., LTD.) (≥6 months from the time of vaccination)
1. meet the new coronary virus pneumonia recovery period diagnostic criteria;
2. short breath, sweating, dry cough, chest tightness, palpitations, etc., three symptoms, or single symptom visual simulation score method (VAS) >=4;
3. the cure discharge is more than 2 weeks;
4. aged 18 ~ 70 years old;

1) Subjects aged 18 years and older;
2) Subjects capable of understanding and voluntarily signing the informed consent;
3) Subjects with general good health as judged by medical history and physical examination. General good health was defined as no significant change of treatment or hospitalization due to disease deterioration within 3 months prior to enrollment;

1.Adult subjects (18 years old), no gender limitation;
2.Subjects in good health or stable clinical situation;
3.Subjects who completed two doses of inactivated COVID-19 vaccine 4-8 months ago;
4.Female subjects of childbearing age are not breast-feeding or pregnant (negative blood pregnancy test), and have no family planning in the first 3 months after enrollment. Take effective contraceptive

1. Individuals aged 18 years and above and are able to provide legal identity certificate. Both male and female should be included;
2 .The subjects shall fully understand the content of the informed consent form and the characteristics of the study vaccine, voluntarily sign the informed consent form, as well as have the ability to use thermometer, scale and fill in diary card and contact card as required;

(1) meet the critical diagnosis criteria for severe COVID-19, (2) are aged between 20 and 80?years and are male or female, (3) have a stable condition and are conscious and cooperative in the examination, (4) volunteer to join the trial and sign the informed consent form, and (5) promise not to perform other exercise programs.

1. Subjects who have received 2 doses of pGX9501 (1.0 mg/2.0 mg) 12 months (+/-2 months) ago;
2. Body mass index (BMI) is between 18-35 kg/m2 (including upper and lower limits);
3. Axillary temperature < 37.3 ℃ on the day of vaccination;
4. Subjects are considered by the investigator as healthy subjects eligible for immunization with this product based on inquiry of medical history and inspection of the physical examination results;

Inclusion Criteria:

Health subjects aged ≥18 years.
Have received two-dose inactivated SARS-CoV-2 vaccine before 6 months or more.
The subject can provide with informed consent and sign informed consent form (ICF)

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Inclusion Criteria:

Male or female adults who are between 18 and 60 years old inclusive
Weigh at least 45kg, with a BMI of 19 to 30 kg/m2 inclusive
No serious underlying disease which would adversely affect the study conduct and data

Inclusion Criteria:

Age ≥ 18 years old,
Through asking medical history and physical examination, the investigator judged that the health condition is good; the chronic disease is stable and there is no change within 3 months of

Inclusion Criteria:

Aged ≥18 years old when signing ICF;
No other anti-SARS-COV-2 vaccines (approved for marketing or registered study) have been previously administered;

Inclusion Criteria:

Adults ≥ 18 years
Negative results of severe acute respiratory syndrome coronavirus 2 (SARS-CoV-2) quantitative reverse transcriptase polymerase chain reaction (qRT-PCR)

① Patients with confirmed mild or common form of novel coronary pneumonia infection. ② Age ≥ 18 years. ③ Clear consciousness, with the ability to express themselves verbally. ④ Those who breathe spontaneously and can cooperate with this experiment.

(1) Inpatients with COVID-19; (2) Those who have received similar antibiotic treatment; (3) Patients who can get out of bed; (4) Individuals who show interest are invited to conduct face-to-face interviews to screen research qualifications.

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Exclusion Criteria

Exclusion criteria: 1. Any situation that makes the programme cannot proceed safely;
2. Known allergy or hypersensitivity reaction to lopinavir / ritonavir;
3. Increase in alanine aminotransferase (ALT) / aspartate aminotransferase (AST) is more than 5 times the upper limit of normal;
4. Use of medications that are contraindicated with lopinavir / ritonavir

Exclusion Criteria:

pregnancy;
patients currently taking corticosteroids (cumulative 400 mg prednisone or equivalent);

Exclusion Criteria:

Age < 14 years or > 80 years;
Pregnant or lactating female;
One of the following items occurred at the enrollment: (i) respiratory failure

Exclusion criteria: 1. Patients are allergic or intolerant to therapeutic drugs;
2. Pregnancy or lactating women;
3. Researchers believe that patients may have other factors affecting the efficacy or safety evaluation of this study.

Exclusion criteria: (1) Patients are diagnosed as a case of severe NCIP;
(2) Patients are pregnant or breastfeeding women;
(3) Patients have aspartate aminotransferase (AST) or alanine aminotransferase (ALT) > 5Ã—UNL or creatinine clearance <50 ml / min;
(4) Patients are allergic or intolerant to the proposed antiviral therapeutic drugs;
(5) Patients are

Exclusion Criteria:

Hypersensitivity to darunavir, cobicistat, or any excipients
Patients with severe liver injury (Child-Pugh Class C)
Concomitant medications that are highly dependent on CYP3A clearance

Exclusion criteria: 1) Severe basic diseases that affect survival, including: uncontrolled malignant tumors with multiple metastases that cannot be removed, hematological diseases, cachexia, active bleeding, severe malnutrition, HIV, etc.;
2) Obstructive pneumonia caused by lung tumors, severe pulmonary interstitial fibrosis, alveolar proteinosis, allergic

Exclusion Criteria:

Disturbance of consciousness
Difficulty swallowing and frequent vomiting
Patients requiring blood transfusion

Exclusion Criteria:

Have a clear history of lopinavir or ritonavir or arbidol allergy
Severe nausea, vomiting, diarrhea and other clinical manifestations affect the oral or absorption of the drugs

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Exclusion Criteria:

Physician makes a decision that trial involvement is not in patients' best interest, or any condition that does not allow the protocol to be followed safely

Exclusion Criteria:

Physician makes a decision that trial involvement is not in patients' best interest, or any condition that does not allow the protocol to be followed safely

Exclusion criteria: 1. Those with allergies;
2. Patients with a history of severe cardio-cerebral vascular disease, diabetes, mental and neurological diseases, severe liver and renal insufficiency;
3. History of hip or knee trauma or surgery;
4. Patients who abuse drugs;
5. Those who are in the acute phase of chronic diseases;
6. Patients with other malignant

Exclusion criteria: 1. Respiratory tract infection caused by primary immunodeficiency disease, acquired immunodeficiency syndrome, congenital respiratory tract malformation, congenital heart disease, gastroesophageal reflux disease, abnormal lung development and other basic diseases, with clear evidence of bacterial infection;
2. Subjects with the following

1. There is clear evidence of bacterial infection in respiratory tract infections caused by basic diseases such as primary immunodeficiency disease, acquired immunodeficiency syndrome, congenital respiratory malformations, congenital heart disease, gastroesophageal reflux disease, and abnormal lung development;

Exclusion criteria: 1. Pregnancy or lactation;
2. Previous serious chronic basic diseases;
3. Previous allergy to traditional Chinese medicine;
4. Newly diagnosed 2019-nCOV pneumonia.

Exclusion criteria: 1) patients were judged not suitable to participate in the study;
2) patients were not suitable to use Chinese patent medicine.

Exclusion criteria: 1) Pregnant or lactating women;
2) Severe basic diseases that affect survival;
3) Other problems thant doctor's judgment is not suitable for the study;
4) Patients have difficulty in taking TCM standard decoctions.

Exclusion criteria: 1. Patients with serious illnesses, such as heart, liver, kidney, endocrine diseases and hematopoietic system disease;
2. Pregnant or lactating women;
3. Patients with mental confusion, with a history of drug abuse or dependence, who allergic to study medication, who have participated in another clinical trial within 3 months or who have other

Exclusion criteria: 1. Patients with serious illnesses, such as heart, liver, kidney, endocrine diseases and hematopoietic system disease;
2. Pregnant or lactating women;
3. Patients who have mental confusion, who with a history of drug abuse or dependence, who allergic to study medication, who have participated in another clinical trial within 3 months or

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Exclusion criteria: 1. Patients with serious illnesses, such as heart, liver, kidney, endocrine diseases and hematopoietic system disease;
2. Pregnant or lactating women;
3. Patients who have mental confusion, who with a history of drug abuse or dependence, who allergic to study medication, who have participated in another clinical trial within 3 months or

Exclusion criteria: 1. Patients with serious illnesses, such as heart, liver, kidney, endocrine diseases and hematopoietic system disease;
2. Pregnant or lactating women;
3. Patients who have mental confusion, who with a history of drug abuse or dependence, who allergic to study medication, who have participated in another clinical trial within 3 months or

Exclusion criteria: 1. Patients with serious illnesses, such as heart, liver, kidney, endocrine diseases and hematopoietic system disease;
2. Pregnant or lactating women;
3. Patients who have mental confusion, who with a history of drug abuse or dependence, who allergic to study medication, who have participated in another clinical trial within 3 months or

Exclusion Criteria:

Patients who meet any of the contraindications in the experimental drug labeling
Patients who do not want to participate in this clinical study

Exclusion Criteria:

Patients who meet any of the contraindications in the experimental drug labeling
Patients who do not want to participate in this clinical study

Exclusion Criteria:

dying state (i.e. survival time is less than 24 hours); progressive malignant tumor with life expectancy less than 6 months; immunosuppressive status(patients using immunosuppressants or

Exclusion criteria: (1) any situation in which the program cannot proceed safely;
(2) allergic constitution;
(3) serious basic diseases affecting survival, including uncontrolled malignant tumors with multiple metastases that cannot be resected, blood diseases, malignant fluid, active bleeding, severe malnutrition, HIV, etc ;
The main organ function is abnormal

Exclusion criteria: (1) Patients are not suitable to take Chinese medicine decoction.
(2) Patients have other serious organ disease or mental illness;
(3) Patients allergy or have contraindications to the drugs involved in the research program.

Exclusion criteria: (1) The critically ill patients in the diagnosis and treatment program of pneumonia with new coronavirus infection issued by the National Health Commission;
(2) It is not suitable for patients who take traditional Chinese medicine decoction;
(3) Those with other serious organ diseases or mental diseases;
(4) Those who are allergic to or have

Exclusion criteria: 1) Any situation that makes the programme cannot proceed safely;
2) patients who have used Kaletra or remdesivir;
3) no clinical or chest imaging manifestations;
4) In the state of no oxygen at rest, the patient's SPO2 <= 94% or the oxygenation index is less than 300mmHg respiratory rate >= 24/min;
5) Patients who need to be

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Exclusion criteria: 1. Allergic history to study medicines;
2. ALT/AST >5 UNL or Child-Pugh Class C;
3. Severely ill patients with life expectance <48 hours;
4. Contraindication of DRV or thymosin;
5. Pregnancy testing positive for child-bearing woman;
6. Known HIV infection;
7. not adequate to be enrolled

Exclusion criteria: 1. Allergic constitution, known to be allergic to baiosavir or farpiravir or pharmaceutical excipients;
2. Body weight <40 kg;
3. Considered as severe disease: has an ongoing respiratory deficiency and subjected to invasive mechanical ventilation; or presence of shock; or admitted to the ICU due to complications;
4. Has known kidney

Exclusion criteria: 1. Hypersensitive to study drug;
2. Body weight <40 kg;
3. Considered as severe disease: has an ongoing respiratory deficiency and subjected to invasive mechanical ventilation; or presence of shock; or admitted to the ICU due to complications;
4. Has known kidney dysfunction determined as CLcr<60 mL/min;
5. Increase in alanine

Exclusion criteria: Exclude patients with severe and critical 2019-nCoV pneumonia

Exclusion criteria: The investigator considers that the subject has other conditions that make him/her unsuitable to participate in the clinical trial or other special circumstances.

Exclusion criteria: (1) diseases need to be differentiated from 2019-ncov, such as influenza virus, parainfluenza virus, adenovirus, respiratory syncytial virus, rhinovirus, human partial pulmonary virus, SARS coronavirus and other known viral pneumonia, mycoplasma pneumoniae, chlamydia pneumonia, bacterial pneumonia and non-infectious diseases;
(2) suspected case of

Exclusion Criteria:

1.2019-nCoV severe Pneumonia patients.

Meet the definition of severe pneumonia(Comply with any of the followings):

Exclusion Criteria:

Allergic to vitamin C;
Dyspnea due to cardiogenic pulmonary edema;
Pregnant or breastfeeding;

Exclusion criteria: 1. Have any known disease that seriously affects the immune system, such as a history of infection with the human immunodeficiency virus (HIV), or malignant tumors of the blood system or solid organs, or splenectomy;
2. Other circumstances that the researcher considers inappropriate to participate in this study

Exclusion criteria: You cannot be enrolled if you meet any of the following criteria:
1) any situation that cannot allow the program to proceed safely;
2) allergic constitution, allergic to more than one kind of food or medicine;
3) patients with severe basic diseases that affect survival, including uncontrolled clinically significant heart, lung, kidney, digestion

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Exclusion criteria: 1. Patients with other malignant tumors requiring treatment;
2. Severe cardiovascular and metabolic diseases beyond the control of combination with other drugs
3. Patients with clinical symptoms of brain dysfunction or serious mental illness who cannot understand or follow the study protocol;
4. Endotracheal intubation with

Exclusion criteria: (1) Immunodeficiency diseases, or those using immunosuppressive agents or glucocorticoids within the past 3 months;
(2) Pregnancies, pregnant women and lactating women;
(3) An allergic condition, such as a history of allergies to two or more drugs or foods, or a known allergy to the ingredients in this test);
(4) Psychotic

Exclusion criteria: 1. Clear evidences?of bacterial infection;
2. Serious primary diseases judged by researchers,such as heart, kidney, lung, endocrine, blood, metabolism and gastrointestinal tract,?may affect the patients' participation in the trial or the outcome of the study;
3. Family history of mental illness or mental illness;
4. Allergic constitution or multi

Exclusion criteria: 1. Severe or critically severe patients;
2. Clear evidence of bacterial infection;
3. Patients with severe primary diseases such as heart, kidney, lung, endocrine, blood, metabolism, and gastrointestinal tract, etc., may affect participation in the trial or may affect the outcome of the study by the judgment of the researcher;
4. Patients

Exclusion criteria: 1. Patients with severe 2019-nCov pneumonia met one of the following conditions: respiratory distress, RR >= 30 times / min; or SaO2 / SpO2 <= 93% in resting state; or arterial partial pressure of oxygen (PaO2) / concentration of oxygen (FiO2) <= 300MMHG (1mmhg = 0.133kpa);
2. Patients with critical 2019-nCoV pneumonia met one of the following

Exclusion criteria: 1. Severe pneumonia requires mechanical ventilationcritically severe cases;
2. Estimated Time of Death is less than 48 hours;
3. There is clear evidence of bacterial infection in respiratory tract infections caused by basic diseases such as primary immunodeficiency disease, acquired immunodeficiency syndrome, congenital

Exclusion Criteria:

Hypersensitivity to chloroquine or hydroxychloroquine;
Women during pregnancy;
Severe heart, lung, kidney, brain, blood diseases or other important systemic

Exclusion criteria: 1. Pregnant or lactating women;
2. There are comorbidities that affect the judgment of the efficacy, such as those with malignant tumors or long-term immunosuppressants;
3. The investigator believes that the patient has other conditions that are not suitable for enrollment;
4. Allergic to dimethyl sulfoxide (DMSO), dextran 40 or

Exclusion criteria: 1. Pregnant or lactating women;
2. Patients who cannot be inhaled by atomization;
3. Patients who are intolerant or allergic to interferon preparations;
4. Other patients were judged unsuitable for inclusion in the trial by researchers.

Exclusion criteria: 1. Known to receive hormone therapy orally or intravenously;
2. Hormone therapy is needed due to concomitant disease upon admission;
3. Patients with diabetes are receiving oral medication or insulin therapy;
4. Known contraindications to dexamethasone or other excipients (such as refractory hypertension;

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Exclusion criteria: 1. Intubation required immidiately in case of cardiac arrest or asphyxia;
2. Pregnancy, severe chronic respiratory, uncontrolled gastric reflux disease and any nasopharyngeal anatomical obstacle;
3. Those patients has intubated in ICU.

Exclusion criteria: Patients who is enrolled in other clinical trial
who had diagnosed as macular degeneration of retinawere or defects of visual field;
who was allergic to 4-aminoquinoline compounds or is prescribed any drug had interactions with hydrocholoroquine
emergency or critically ill patients who can not received oral drugs
pregnancy or lactation

Exclusion criteria: (Subjects cannot enter the study if they meet any of the following conditions):
1. Patients with a history of allergy to chloroquine phosphate, lopinavir, and ritonavir;
2. Patients with hematological diseases;
3. Patients with chronic liver and kidney disease and reaching the end stage;
4. Patients with arrhythmia and chronic heart disease;

Exclusion criteria: 1. Pregnant or lactating women, or pregnant;
2. Those who are allergic to the drug ingredients of this program;
3. Severe cardiovascular diseases, such as severe center of gravity dysfunction, second or third degree atrioventricular block, etc.;
4. Severe hepatic insufficiency: defined as the upper limit of ALT > 2 times normal value or AST

Exclusion criteria: Patients over 80 years of age or less than 12 years of age (trial 3 and less than 18 years old); patients with contraindications to study drugs; cancer patients, life expectancy <6 months; participants in other clinical trials in the past three months; pregnant women, breastfeeding Women

Exclusion criteria: 1. Patients with severe pneumonia requiring mechanical ventilation and severe new coronavirus infection with pneumonia;
2. Death is expected within 48 hours;
3. There is clear evidence of bacterial infection in respiratory tract infections caused by primary immunodeficiency disease, acquired immunodeficiency syndrome, congenital

Exclusion criteria: 1. Chronic lung diseases requiring long-term oxygen therapy;
2. Long-term application of glucocorticoids due to underlying diseases;
3. Based on the researcher's judgment, previous or current diseases may affect patients' participation in the trial or influence the outcome of the study, including: malignant disease, autoimmune disease

Exclusion criteria: 1. The doctor believes that the patient is not suitable to participate in this trial, including those who may not cooperate, do not comply with the requirements of the procedure, or participate in this trial may put the patient in an unsafe situation.
2.Pregnant or lactation periods women;
3.Immunoglobulin allergy;
4.Immunoglobulin A deficiency;

Exclusion criteria: 1. Severe liver disease (such as child Pugh score >= C, AST > 5 times upper limit);
2. The patients were allergic to the components in asc09 / ritonavir compound tablets;
3. Patients with contraindications specified in the instructions of ritonavir tablets;
4. The pregnancy test of female subjects in the screening period was positive;

Exclusion criteria: 1. Known allergic to hydroxychloroquine or 4-aminoquinoline;
2. Severe renal impairment (eGFR <= 30 mL/min/1.73 m2), and those with RRT;
3. Severe liver disease (Child Pugh score >= C, AST>5 times upper limit);
4. Pre-existing retinopathy of the eye;
5. History of epilepsy;
6. G6PD deficiency;
7. Pregnant or

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Exclusion criteria: Patients with any of the following were excluded from the study:
1. Patients with previous allergy to 4-aminoquinoline compounds;
2. Patients with previous severe retinal or visual field related diseases;
3. Patients with severe liver dysfunction and patients with renal dysfunction (creatinine clearance rate <= 50 mL / min);
4. Patients

Exclusion criteria: Patients with any of the following were excluded from the study:
1. Patients with previous allergy to 4-aminoquinoline compounds;
2. Patients with previous severe retinal or visual field related diseases;
3. Patients with severe liver damage;
4. Patients with impaired renal function (creatinine clearance <=50 mL / min);
5. Patients weighing

Exclusion criteria: (1) Patients who can not guarantee compliance of using TCM during the treatment period, or patients who are difficult to take medicine by oral or nasal route.
(2) Patients who have been more than 7 days since definite diagnosis.
(3) Patients with severe primary respiratory disease or other pathogenic microbial pneumonia that needs to

Exclusion criteria: 1. Patients who are participating in other drug clinical trials;
2. Pregnant or lactating women;
3. ALT / AST> 5 ULN, neutrophils <0.5, platelets less than 50;
4. Definite diagnosis of rheumatic immune-related diseases;
5. Long-term oral anti-rejection or immunomodulatory drugs;
6. Hypersensitivity to tocilizumab or any excipients;
7.

Exclusion criteria: 1. Infected with other viruses such as HCV, HIV, and syphilis;
2. Used non-steroidal anti-inflammatory drugs within 3 days before enrollment;
3. Have severe liver disease;
4. Those who are known to be allergic to the test drug and its ingredients;
5. Any of the following risk factors:
1) Chronic respiratory diseases such as chronic

Exclusion criteria: Children, pregnant women

Exclusion criteria: 1. Severe pneumonia requires mechanical ventilationcritically severe cases;
2. Estimated Time of Death is less than 48 hours;
3. There is clear evidence of bacterial infection in respiratory tract infections caused by basic diseases such as primary immunodeficiency disease, acquired immunodeficiency syndrome, congenital

Exclusion criteria: 1. Pregancy or lactating women;
2. People with allergies or allergies to components of Truncated Torsion formula;
3. Patients with serious basic diseases such as malignant tumor, cirrhosis, chronic renal failure (uremic stage), hematological disease, HIV, etc.;
4. Patients with obstructive pneumonia caused by lung tumor, pulmonary

Exclusion criteria: 1)One of the following conditions:
Respiratory distress, RR >= 30 times/min;
At rest, oxygen saturation is less than 93%;
Artial arterial oxygen pressure (PaO2)/oxygen absorption concentration (FiO2) <= 300mmHg (1mmHg=0.133kPa);
Respiratory failure is present and mechanical ventilation is required;
Shock;

Exclusion criteria: 1)One of the following conditions:
Respiratory distress, RR >= 30 times/min;
At rest, oxygen saturation is less than 93%;
Atrial arterial oxygen pressure (PaO2)/oxygen absorption concentration (FiO2) <= 300mmHg (1mmHg=0.133kPa);
Respiratory failure is present and mechanical ventilation is required;
Shock;

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Exclusion criteria: 1. People with allergies and allergies to experimental drugs;
2. Combined with other malignant tumors;
3. Patients with hematological diseases, coagulopathy and autoimmune diseases;
4. Combined with severe cardiovascular diseases, Patients with vascular disease, hematopoietic system disease, and neuropathy;
5. Participated

Exclusion criteria: 1. Patients with cognitive disorder or known mental illness;
2. Pregnant or lactating women;
3. Patients with active tuberculosis, pulmonary abscess or chronic respiratory failure;
4. Patients with tumor;
5. Patients with severe liver or kidney diseases;
6. Patients with severe cardiovascular or cerebrovascular diseases;
7. Patients who are

Exclusion criteria: (1) Impaired consciousness or mental illness cannot cooperate with treating the observer.
(2) Those with severe organ dysfunction.
(3) Pregnant women.

Exclusion Criteria:

1. Pregnancy or breastfeeding
2. Known HIV, HBV or HCV infection
3. Patients with malignant tumor, other serious systemic diseases and

Exclusion criteria: 1. The physician decides that participating in the trial is not in the best interest of the patient, or does not meet any conditions for safe follow-up in the protocol;
2. People with heart disease such as cardiac block;
3. People with eye diseases such as retina and cornea;
4. Severe liver disease (such as Child Pugh score >= C or AST > 5 times the

Exclusion criteria: 1. Aged <18 years;
2. Pregnant or lactating women;
3. Allergic to the test drug;
4. The underlying disease is very serious, and the expected survival time is less than 6 months (such as advanced malignant tumors);
5. COPD or end-stage lung disease requires home oxygen therapy;
6. Expected survival time does not

Exclusion criteria: (1) Immunodeficiency disease, or those who have used immunosuppressive agents or glucocorticoids within the past 3 months;
(2) Preparing pregnant, pregnant and lactating women;
(3) Patients with allergies (referring to allergies to two or more medicines, foods or known ingredients in this study);
(4) Patients with mental illness, or have no

Exclusion criteria: (1) Those who cannot take Chinese traditional medicine decoction;
(2) mentally ill subjects who are not easy to control;
(3) those who are pregnant or breast-feeding;
(4) those who use other Chinese medicines;
(5) those who are not considered suitable to participate in this trial by researchers.

Exclusion criteria: If subject meet one of the following exclusion critera, he/she cannot be enrolled in this study:
1. Be dysphagia and cannot receive drud by oral administration;
2. Complicated with pneumonia infected by other virus;
3. Allergy to or intolerance to study drug phosphoric chloroquine;
4. With significantly abnormal liver function(

Exclusion Criteria:

Patients with severe allergies or allergies to stem cell preparations and their components;

Patients with serious basic diseases that affect survival, including: blood

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Exclusion Criteria:

ALT≥5 times of ULN, level of total bilirubin≥3 times of ULN, or Cr≥1.5 times of ULN.

Patients with serious severe liver disease

Exclusion criteria: 1. Serious/critically ill patients with novel coronavirus(COVID-19) pneumonia or patients that clinician consider that he/she shall be treated in ICU;
2. With pneumonia infected by other virus;
3. With one of following abnormal laboratory parameters(judged per local Laboratory reference range):
-ALT or AST≥5 ULN or;
-ALT

Exclusion criteria: Participants who have used fluoroquinolone antibiotics within 1 week

Exclusion criteria: 1. Involving in other drug clinical trial;
2. Pregnant or breastfeeding;
3. Presence of other serious diseases that may be life threatening, such as cancer;
4. Expected life is less than 24 hours;
5. Allergy to ALA or drugs of similar chemical classes (Vitamin B) previous history of intolerance to recommended target doses of alpha-LA;

Exclusion criteria: (1) known or suspected allergy to the components of Azivudine tablets;
(2) patients with malabsorption syndrome or any other condition that affects gastrointestinal absorption and requires intravenous nutrition or unable to take oral medication;
(3) patients who is currently receiving anti-hiv treatment;
(4) patients with one of the following

Exclusion criteria: 1) pneumonia patients with severe or critical new coronavirus infection;
2) daily treatment of asthma, any other chronic respiratory disease;
3) patients with confirmed history of diabetes and HbA1C >=8.0%;
4) those allergic to the known ingredients of the drug;
5) pregnant or nursing women;
6) patients with severe mental

Exclusion criteria: 1. Patients who have received immunotherapy (such as PD-1 / L1, CTLA4, etc.) and inflammatory factor modulators (such as Ulinastatin) within one month.
2. Patients who have used antibacterial drugs such as macrolides within one week.
3. Patients who have received other anti-2019-nCOV potentially effective drugs such as Lopinavir

Exclusion criteria: 1. Aged less than 18 years old;
2. Other serious medical diseases such as malignant tumor, heart, liver and kidney disease, uncontrollable metabolic disease, etc;
3. Not suitable for gastrointestinal administration;
4. Pregnant or lactating women;
5. Those who are allergic to the ingredients of this product;
6. Mental state can not

Exclusion criteria: 1. Pregancy or lactating women;
2. People with allergies or allergies to traditional Chinese medicine
3. Severe Primary disease such as uncontrolled and metastasis malignancy, hematopathy, HIV;
4. Obstructive pneumonia caused by lung tumors, severe pulmonary interstitial fibrosis, alveolar proteinosis, allergic reaction alveolitis;
5

Exclusion Criteria:

severe immunosuppression (HIV infection, long-term use of immunosuppressive agents

pregnant or lactation period women

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Exclusion criteria

Patients who have undergone lung surgery that affects pulmonary function, such as pulmonary transplantation, pulmonary resection, pulmonary volume reduction, etc;

Exclusion criteria: 1 Patients with identified allergies to 4-aminoquinoline and lopinavir/litonavir;
2 Patients with server diseases of the blood system;
3 Patients with identified severe liver disease, underlying cirrhosis or alanine aminotransferase (ALT)/aspartate aminotransferase (AST) elevation more than 3 times the normal upper limit;
4 Patients with alcoholism or

Exclusion criteria: 1. Patients with identified allergies to 4-aminoquinoline and lopinavir/litonavir;
2. Patients with server diseases of the blood system;
3. Patients with identified severe liver disease, underlying cirrhosis or alanine aminotransferase (ALT)/aspartate aminotransferase (AST) elevation more than 3 times the normal upper limit;
4. Patients with alcoholism or

Exclusion criteria: 1. Female patients during pregnancy;
2. Patients with known allergy to chloroquine;
3. Patients with haematological diseases;
4. Patients with chronic liver or kidney diseases at the end stage;
5. Patients with arrhythmia and chronic heart disease;
6. Patients with retinal disease, hearing deficiency or hearing loss;
7. Patients with

Exclusion criteria: 1. Patients with respiratory failure, acute necrotizing encephalopathy, septic shock, multiple organ dysfunction, and other serious clinical conditions requiring monitoring and treatment when enrolled;
2. Pregnant, preparing or lactating women;
4. Patients with severe heart, cerebrovascular and liver, kidney, hematopoietic system diseases and

Exclusion criteria: 1. Patients with respiratory failure, acute necrotizing encephalopathy, septic shock, multiple organ dysfunction, and other serious clinical conditions requiring monitoring and treatment when enrolled;
2. Pregnant, preparing or lactating women;
3. Patients with severe heart, cerebrovascular and liver, kidney, hematopoietic system diseases and

Exclusion criteria: 1. There is clear evidence of bacterial infection in respiratory tract infection caused by basic diseases such as primary immunodeficiency disease, congenital respiratory tract malformation, congenital heart disease, gastroesophageal reflux disease and pulmonary dysplasia;
2. Subjects with the following conditions: asthma requiring

Exclusion criteria: No

(1) Positive for other pathogens such as A, B, Mycoplasma, Adenovirus;
(2) patients require mechanical ventilation;
(3) People with metal or pacemaker;
(4) active tuberculosis;
(5) Massive pulmonary fibrosis;

Exclusion criteria: 1. The serious NCP patients who can neither swallow medicines nor take medicine via stomach tube, and the critical serious NCP patients;
2. Those with severe hypo-immunity due to, but not limited to, one of the following reasons:
1) HIV/AIDS, the condition is still unstable in the process of mediation and /or the last detection value of CD4 plus T

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Exclusion criteria: 1. Patients with other serious diseases such as lung diseases, cardiovascular and cerebrovascular diseases, hematopoietic diseases, autoimmune diseases, digestive system, or mental illness;
2. Pregnant or lactating women;
3. Participate in other forms of exercise during the trial

Exclusion criteria: 1) Cases with severe vomiting and difficulty in taking drugs orally or causing difficulty in taking drugs after oral administration;
2) Pregnant and lactating women;
3) The subjects received specific antiviral drugs such as lopinavir / lidonavir, ribavirin and monoclonal antibody within one week before admission;
4) Cases with no hope of survival

Exclusion criteria: 1. critically ill patients based on the New Protocol for Diagnosis and Treatment of NCP;
2. Patients with eye diseases (such as cataract, conjunctivitis, glaucoma)
3. Severe liver disease (such as AST > 3 ULN);
4. Patients with severe renal insufficiency (estimated glomerular filtration rate <=30ml/min/1.73m2) or continuous renal

Exclusion criteria: 1. Malignant disease in the past five years;
2. The clinical prediction is that there is no hope for survival, only cases of hospice care, or cases of deep coma that do not respond to supportive treatment measures within three hours of admission;
3. Subjects who are participating in other clinical trials or within 3 months;
4. Cases of very

Exclusion criteria: 1. Expected to die within 48 hours;
2. Patients with asthma attack, purulent tonsillitis, acute and chronic bronchitis, sinusitis, otitis media, and other respiratory diseases affecting clinical trial evaluation; and chest CT confirmed the existence of severe pulmonary interstitial disease, bronchiectasis, obstructive pulmonary disease With lung disease;
3.

Exclusion criteria: 1. In the opinion of the researchers, the participation of the trial is not in the best interests of the patients, or any situation is not in line with the situation that the trial scheme is allowed to be followed safely;
2. According to the judgment of the investigator, the diseases previously or now may affect the patients' participation in the trial or the outcome of the study

Exclusion criteria: 1. Critical case with the following situation:
 i. Patients with respiratory failure and needing mechanical ventilation;
 ii. Patients with shock;
 iii. Patients with other organ failures and needing ICU monitoring and treatment;
2. Patients who have other respiratory tract infection;
3. Patients used Liu Shen Capsule within 1 month before

Exclusion criteria: 1. Patients with other serious diseases such as lung diseases, cardiovascular and cerebrovascular diseases, hematopoietic diseases, autoimmune diseases, digestive system, or mental illness;
2. Pregnant or lactating women;
3. Participate in other forms of exercise during the trial

Exclusion criteria: (1) The PI considers that the patient is not suitable to participate the study or the patient may be unsafe if he participate the study;
(2) Patients with serious liver disease, grade C according to child-pugh;
(3) Patients with severe renal impairment (glomerular filtration rate <=30ml /min(1.73m2) or who have received continuous renal replacement

Exclusion criteria: 1. Patients who are participating in other drug clinical trials;
2. Pregnant or lactating women;
3. ALT / AST> 5 ULN, neutrophils <0.5, platelets less than 50;
4. Expected life<1 week;
5. Definite diagnosis of asthma-related diseases;
6. Long-term oral anti-rejection or immunomodulatory drugs;
7. Hypersensitivity to tocilizumab or any

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Exclusion criteria: No

Exclusion criteria: (1) Hyperthyroidism;
(2) Favism;
(3) Pregnancy;
(4) Abnormal coagulation, thrombocytopenia, active bleeding;
(5) People with medical ozone allergy or hypersensitivity;
(6) Severe anemia;
(7) Hypocalcemia;
(8) Unstable period of severe cardiovascular disease;
(9) Uncontrolled diabetes

Exclusion criteria: 1. Combining basic diseases such as hypertension, diabetes, and coronary heart disease;
2. Malignant tumor or with a history of malignant tumor;
3. pregnant or lactating women;
4. suffer from severe mental illness;
5. History of unstable angina pectoris or myocardial infarction within 6 months; coronary angiography suggested

Exclusion criteria: 1. The clinical classification of patients with severe novel coronavirus infection is to meet any of the following:
1) Respiratory failure occurs and requires mechanical ventilation;
2) Shock occurs;
3) Combined failure of other organs requires ICU monitoring and treatment;
2. Those who are allergic to blood products or plasma

Exclusion criteria: 1. Pregnant women;
2. Patients with a history of angina pectoris, myocardial infarction and other serious cardiovascular diseases;
3. The researchers judged that this study was not in the interests of patients.

Exclusion Criteria:

Allergies, known to be allergic to research drugs or drug excipients;
Patient weight is less than 40 kg;
Respiratory distress-RR>30 / min on room air or SPO2< 93%, or PaO2/FiO2

Exclusion criteria: 1. Females who are pregnant or likely to be pregnant, or who are lactating and unable to stop breastfeeding, or who have positive pregnancy tests during screening;
2. Males or females who have a birth plan or are unwilling to take reliable contraceptive measures for contraception within 90 days from signing the informed consent to the last

Exclusion criteria: Exclude patients with influenza, adenovirus and other upper respiratory tract infections

Exclusion criteria: (1) Patients with known or suspected to be allergic to the components of the test drug;
(2) Patients with known hepatitis B, C, AIDS and other viral infections;
(3) Patients with severe or severe clinical classification who have experienced complications such as respiratory failure and acute respiratory distress syndrome (ARDS);
(4)

Exclusion criteria: (1) Subjects who have any active autoimmune disease or history of autoimmune disease;
(2) Subjects who received immunosuppressive therapy by using immunosuppressant, or systemic or absorbable topic hormone therapy (temprednisone or other therapeutic hormones with a dosage >10 mg/day) within 14 days prior to the

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Exclusion criteria: 1. Serious/critically ill patients with novel coronavirus(COVID-19) pneumonia or patients that clinician consider that he/she shall be treated in ICU;
2. With pneumonia infected by other virus;
3. With one of following abnormal laboratory parameters(judged per local Laboratory reference range):
-ALT or AST>5 ULN or;
-ALT

Exclusion criteria: 1. The physician decides that participating in this study is not in the best interest of the subject or that there exist a circumstance in which the agreed protocol cannot be safely followed;
2. People who have allergic reactions or allergies to antiviral oral solution or Wuzhifangquanfang decoction;
3. Those who do not meet the national

Exclusion criteria: (1) Diagnosed with other known viral pneumonia,like influenza virus, parainfluenza virus, adenovirus, respiratory syncytical virus, rhinovirus, human metapneumovirus and SARS coronavirus, etc.;
(2) Diagnosed with mycoplasma pneumonia, chlamydial pneumonia and bacterial pneumonia;
(3) Diagnosed with vasculitis, dermatomyositis

Exclusion criteria: 1. Pregnancy or lactating women;
2. People with allergies or allergies to Shenfu injection and its components;
3. Severe basic diseases that affect survival, including: uncontrolled malignant tumors with multiple metastases that cannot be removed, hematological diseases, cachexia, active bleeding, severe malnutrition, HIV, etc.;
4. Obstructive

Exclusion Criteria:

Age <18 years; Age >80 years
Pregnant or breast feeding woman or with positive pregnancy test result
P/F <100 mmHg

Exclusion criteria: 1. Investigator makes a decision that trial participation is not in patients' best interest, or any condition that does not allow the protocol to be followed safely;
2. As judged by investigator historic or current diseases that may have impact on the subjects participation of the study or the study outcome. These diseases include but not limited to cadiac diease

Exclusion criteria: 1. Patients with presence, coagulopathy or hypotension;
2. Pregnant and lactating women;
3. People with allergies or allergies to Xuebijing Injection and its components;
4. Severe basic diseases that affect survival, including: uncontrolled malignant tumors that have metastasized and cannot be removed, blood diseases, cachexia, active

Exclusion criteria: 1. severe vomiting and difficulty in taking oral medication or absorbing in oral medication;
2. pregnant and nursing women;
3. subjects received lopinavir/ridonavir, ribavirin, monoclonal antibodies and other specific antiviral drugs in the week prior to admission;
4. subjects exist respiratory failure requiring mechanical ventilation;
5. subjects of

Exclusion criteria: The investigator considers that the subject has other conditions that make him/her unsuitable to participate in the clinical trial or other special circumstances.

Exclusion criteria: 1. In the six months before the diagnosis of 2019-nCoV acute respiratory disease, the patient was clearly diagnosed as a psychiatric disorder, including depression, bipolar disorder and so on;
2. Patients with psychotic symptoms;
3. A history of suicide attempt in the 6 months before the study, or a score of more than 3 on item 3 (suicide item) of the

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Exclusion criteria: No

Exclusion criteria: 1. Patients who are participating in other drug clinical trials;
2. Pregnant or lactating women;
3. ALT / AST> 5 ULN, neutrophils <0.5, platelets less than 50;
4. Expected survival is less than 1 week;
5. Patients diagnosed with rheumatic immune related diseases;
6. Take orally anti rejection drugs or immunomodulatory drugs for a long time;

Exclusion criteria: 1. Any situation that does not allow the program to proceed safely;
2. The patient may be transferred to a non-participating hospital within 72 hours;
3. Those who have continued to use immunosuppressive agents or organ transplants for the past 6 months;
4. Glucose-6-phosphate dehydrogenation Enzyme deficiency (faba bean

Exclusion criteria: 1) ARDS caused by other viral infections;
2) ARDS caused by other non-viral infections (such as trauma);
3) WHO grade III or IV pulmonary hypertension;
4) Patients with malignant tumors of the lungs and other systems;
5) Positive blood pregnancy test in lactating women or screening women;
6) Idiopathic IgA deficiency;
7)

Exclusion criteria: 1. Allergic constitution (persons who are allergic to more than 2 kinds of substances), people who are allergic to similar drugs in the past or those who are known to be allergic to the ingredients and auxiliary materials in the test drugs;
2. Patients who have participated in other clinical trials in the past 3 months;
3. Patients not suitable for the clinical

Exclusion criteria: (1) with severe liver or kidney failure or other serious underlying diseases;
(2) Pregnant and lactating women;
(3) Persons who are participating in other clinical trials.

Exclusion criteria: 1. Allergic to vitamin C;
2. Cardiogenic dyspnea;
3. Life expectancy<24h;
4. Pregnant and/or lactating female;
5. There is a state of tracheotomy or home oxygen therapy in the past;
6. Previously complicated with interstitial lung disease, malignant tumors diffuse alveolar hemorrhage, diabetic ketoacidosis, or an active

Exclusion criteria: Subjects who meet one of the following conditions will not be eligible for the trial:
1. pregnant or lactating women;
2. Malignant tumors, patients with other severe systemic diseases and mental illness;
3. Those who have recently participated in other clinical trials;
4. There is evidence of drug addiction within one year before joining the trial;

Exclusion criteria: 1. The patient has other immune system, blood system, heart, liver, kidney and other important organs of the disease;
2. Accompanied by chronic diseases, such as malignant tumors, chronic obstructive pulmonary disease;
3. have a history of mental illness;
4. pregnant and nursing women;
5. less than 18 years old;
6

Exclusion criteria: (1) Immunodeficiency disease, or those who have used immunosuppressive agents or glucocorticoids within the past 3 months;
(2) Preparing pregnant, pregnant and lactating women;
(3) Patients with allergies (referring to allergies to two or more medicines, foods or known ingredients in this study);
(4) Patients with mental illness, or have no

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Exclusion criteria: 1. History of tumor;
2. Vascular embolism and pulmonary hypertension;
3. Suspected or established history of alcohol, drug or drug abuse;
4. Female patients of child-bearing age who are pregnant, breast-feeding or have a recent family planning;
5. Patients who have participated in or are participating in other drug trials within one

Exclusion criteria: 1) Any situation where the solution cannot be carried out safely;
2) Allergic constitution, allergic to plasma or drugs;
3) Being too old with severe underlying diseases that affect survival, including uncontrolled clinically significant heart, lung, kidney, digestive, hematological, neuropsychiatric, immune, metabolic, or malignant tumors. Severe

Exclusion criteria: 1. Patients who are participating in clinical trials of other drugs;
2. Pregnant or lactating women;
3. ALT / AST > 2-fold ULN;
4. Expected survival time < 1 week;
5. With serious liver disease;
6. Patients taking orally anti-tumor drugs or immunomodulatory drugs for a long time;
7. Patients who are participating in other clinical trials;

Exclusion criteria: (1) It is difficult to administer to patients by oral or nasal feeding;
(2) Pregnant women, as well as patients with other malignancies such as malignant tumors, mental illnesses, and patients who are not considered suitable for study.

Exclusion Criteria:

Pregnancy, lactation and those who are not pregnant but do not take effective contraceptives measures;
Patients with malignant tumor, other serious systemic diseases and

Exclusion Criteria:

Patients with severe allergies or allergies to stem cells;
Patients with serious basic diseases that affect survival, including: blood diseases, cachexia, active bleeding, severe malnutrition, etc ;

Exclusion criteria: 1. Pneumonia caused by other reasons;
2. Have contraindications that can not participate in sports activities;
3. Mental disorders, cognitive impairments etc. can not cooperate with researchers subjectively.

Exclusion criteria: 1) Any situation where the solution cannot be carried out safely;
2) Allergic constitution, allergic to one or more foods or drugs;
3) Severe basic diseases affecting survival, including uncontrolled clinically significant heart, lung, kidney, digestive, hematological, neuropsychiatric diseases, immune diseases, metabolic diseases or

Exclusion criteria: (1) Meet the diagnostic criteria for severe new-type coronavirus infection pneumonia;
(2) Severe primary diseases that affect survival, including: uncontrolled malignant tumors, hematological diseases, and HIV that have not been metastasized in multiple places;
(3) Obstructive pneumonia, pulmonary interstitial fibrosis, alveolar proteinosis

Exclusion criteria: 1. Allergy to drugs and excipients used in this study;
2. Pregnant and lactating women and women of child-bearing years did not agree to use contraception during the experiment;
3.Have malignant tumor or hypercoagulant state;
4. Suffer from mental illness and can't cooperate well;
5. Those who have participated in or are participating in

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Exclusion criteria: 1. Patients with other serious pulmonary diseases, cardiovascular and cerebrovascular diseases, diabetes mellitus, tumor, hematopoiesis, autoimmune diseases, digestive system diseases, skin diseases, psychosis, etc.;
2. Patients who cannot complete the treatment;
3. Pregnant or lactating women

Exclusion criteria: (1) Allergic to fabiravir or abidol;
(2) ALT/AST increased 5 times higher than the upper limit of normal, or with child Pugh C;
(3) Severe patients with expected survival time < 48 hours;
(4) Pregnancy;
(5) HIV positive;
(6) Considered unsuitable by researchers

Exclusion criteria: 1. There is clear evidence of bacterial infection in respiratory tract infections caused by basic diseases such as primary immunodeficiency disease, acquired immunodeficiency syndrome, congenital respiratory malformations, congenital heart disease, gastroesophageal reflux disease, and abnormal lung development;
2. Subjects with the following

Exclusion criteria: 1. The pneumonia patients with severe new coronavirus infection met one of the following conditions: respiratory distress, RR <=30 times / min; or SaO2 / SpO2 <=93% in resting state; or arterial partial pressure of oxygen (PaO2) / concentration of oxygen (FiO2) <=300MMHG (1mmhg = 0.133kpa);
2. Pneumonia patients with severe new

Exclusion criteria: 1. No malnutrition;
2. Patients who had received antiviral drugs, immunosuppressive drugs and cytotoxic drugs 2 weeks before diagnosis;
3. patients with hypertriglyceridemia (TG): Serum TG > 3.0mmol/l;
4. patients with severe liver and kidney dysfunction;
5. pregnant women; 6 tumor patients

Exclusion criteria: (1) Age < 18 or > 65;
(2) Patients with severe heart, brain, lung, kidney dysfunction, endocrine disease, hematopoiesis system disease or other serious diseases and psychosis;
(3) Pregnant and lactating women.

Exclusion criteria: 1. Serious basic diseases affecting survival, including: uncontrolled malignant tumor with multiple metastases that cannot be removed, hematological diseases, cachexia, active hemorrhage, severe malnutrition, AIDS, etc.;
2. Obstructive pneumonia, severe pulmonary interstitial fibrosis, alveolar proteinosis and allergic alveolitis caused by lung

(1) Critical patients.

(2) Patients who can not guarantee compliance of using TCM during the treatment period, or patients who are difficult to take medicine by oral or nasal route.

(3) Patients with severe primary respiratory disease or other pathogenic

Exclusion criteria: 1. AST and ALT> 1.5 x ULN at visit 1;
2. bilirubin> 1.5 x ULN at visit 1;
3. At visit 1, calculated by Cockcroftâ€“Gault formula, creatinine clearance <30 mL / min;
4. Patients with potential chronic liver disease (Child Pugh A, B or C liver injury);
5. Previous treatment with nidanib or pirfenidone;
6. One month before the screening visit (Visit 1)

Exclusion criteria: 1. Patients undergoing general gastroscopy;
2. With trachea dysfunction;
3. pregnant women, children, age more than 80 years;
4. Patients with difficulty in intubation.

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Exclusion criteria: 1.Serious basic diseases affecting survival, including: uncontrolled malignant tumor with multiple metastases that cannot be removed, hematological diseases, cachexia, active hemorrhage, severe malnutrition, AIDS, etc.;
2.Obstructive pneumonia, severe pulmonary interstitial fibrosis, alveolar proteinosis and allergic alveolitis caused by lung

Exclusion criteria: Doctors and researchers consider that the patient inappropriate to enroll in the study.

Exclusion Criteria

Cohort 1 and 2:

Subjects who meet any of the following exclusion criteria are not to be

Exclusion criteria: 1. The clinical classification of patients with severe novel coronavirus infection is to meet any of the following:
1) Respiratory failure occurs and requires mechanical ventilation;
2) Shock occurs;
3) Combined failure of other organs requires ICU monitoring and treatment;
2. Those who are allergic to blood products or plasma

Exclusion criteria: 1. Those who are allergic to Xuebijing injection and its components;
2. Pregnant or lactating women;
3. Serious basic diseases affecting survival, including: malignant tumor, hematopathy, HIV, etc;
4. Expected death within 48 hours;
5. Patients who are participating in other clinical trials;
6. The researchers think other

Exclusion criteria: 1. Cannot take medicine by mouth;
2. Those who are allergic to bismuth potassium citrate capsules or who have taken bismuth preparations within 1 month;
3. Critical patients meet one of the following conditions:
(1) Respiratory failure occurs and requires mechanical ventilation;
(2) Shock occurs;
(3) ICU monitoring and

Exclusion criteria: 1. Patients with allergic history to 4-aminoquinoline (such as chloroquine, hydroxychloroquine, piperaquine, aminophenquine, etc.) and similar drugs (such as primaquine, etc.) were identified;
2. Patients with blood system diseases;
3. Patients with hearing impairment;
4. Patients with known severe liver disease, basic disease of liver cirrhosis or

Exclusion criteria: 1. Heart rate: Heart rate <40 or> 130 beats / min;
2. New onset of myocardial ischemia or arrhythmia;
3. Blood pressure: mean arterial pressure (MAP) <65mmHg or> 110mmHg;
4. Body temperature: >=38.5 degree C;
5. Blood oxygen saturation <=93%;
6. Hemodynamic instability;
7. Severe liver and kidney dysfunction

Exclusion criteria: 1) patients in coma, unable to carry out voluntary activities and social activities;
2) patients with mental illness or adverse reactions caused by drugs;
3) patients with serious diseases such as cardiovascular, cerebrovascular, liver and kidney damage.

Exclusion criteria: Patients with severe cardiovascular disease, lung disease, sleep disorder, psychological disease, musculoskeletal system disease and other sports contraindications.

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Exclusion criteria: 1. Intubation required immidiately in case of cardiac arrest or asphyxia; 2. Pregnancy, severe chronic respiratory, uncontrolled gastric reflux disease and any nasopharyngeal anatomical obstacle; 3. Those patients has intubated in ICU.

Exclusion criteria: 1) patients in coma, unable to carry out voluntary activities and social activities;
2) patients with mental illness or adverse reactions caused by drugs;
3) patients with serious diseases such as cardiovascular, cerebrovascular, liver and kidney damage.

Exclusion criteria: 1. Patients with mild and critical novel coronavirus pneumonia;
2. Acute respiratory disease caused by non- novel coronavirus pneumonia;
3. Patients with diabetes and hypertension accompanied by severe primary immunodeficiency disease acquired immunodeficiency syndrome congenital respiratory malformation congenital

Exclusion criteria: (1) Female patients during pregnancy or lactation;
(2) Patients allergic to lipoic acid or similar drugs (B vitamins), previously intolerant to the recommended dose of lipoic acid;
(3) Patients who are taking part in clinical trials of other drugs;
(4) Patients are taking other foods or drugs with antioxidant effect, such as vitamin C, vitamin E, etc

Exclusion criteria: (1) patients with severe heart, liver, kidney and other complications or other serious primary disease or mental illness;
(2) patients whose survival time is not expected to exceed 48 hours from screening;
(3) endotracheal intubation and mechanical ventilation had been performed during screening;
(4) any influence on inclusion judged

Exclusion criteria: 1) Patients with mild, severe or critical novel coronavirus disease;
2) According to the researcher's judgment, previous or current diseases may affect patients' participation in the trial or influence the outcome of the study, including: malignant disease, autoimmune disease, severe malnutrition, liver and kidney disease, blood disease, nervous system

Exclusion criteria: 1) Patients with mild, severe or critical novel coronavirus disease;
2) According to the researcher's judgment, previous or current diseases may affect patients' participation in the trial or influence the outcome of the study, including: malignant disease, autoimmune disease, severe malnutrition, liver and kidney disease, blood disease, nervous system

Exclusion criteria: 1. Any situation where the programme cannot be carried out safely;
2. Patients who have used interferon or remedesivir;
3. No clinical manifestations and chest imaging findings;
4. Known allergy or hypersensitivity to interferon (including asthma);
5. Disabled in patients with uncontrolled autoimmune diseases;
6. Patients with severe

Exclusion criteria: 1. Patients who are allergic to corticosteroid;
2. Patients who are diagnosed with adrenal insufficiency;
3. Severe immunosuppression, one of the following: infection with human immunodeficiency virus and CD4 cell count below 350 cells per microliter, immunosuppressive therapy after solid organ transplantation, neutropenia

Exclusion criteria: (1) asthma requiring daily treatment, any other chronic respiratory disease
(2) patients with severe mental illness and severe neurosis.?
(3) the risk stratification of exercise rehabilitation was assessed as high-risk patients or those who were restricted from activities for other reasons;
(4) patients with severe renal insufficiency and liver

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Exclusion criteria: ? Doctors decided that it was not in the best interests of the subjects to participate in the study, or there was any situation that could not follow the agreement safely; ? there was a history of pulmonary embolism; ? there were patients with human immunodeficiency virus (HIV) infection; ? there were severe organ failure; such as severe liver disease

Exclusion Criteria:

With active tuberculosis, idiopathic pulmonary fibrosis, bronchial asthma, bronchiectasis, pulmonary embolism, patients with chronic respiratory failure or other severe respiratory disease;

Exclusion criteria: 1. Sever cases with shock, acute respiratory distress syndrome;
2. Multiple organ failure;
3. Pregnant or lactating women;
4. Cases with malignant disease, autoimmune disease or sever diseases of immune immune system, such as: HIV infection;
5. Major organ transplantation;
6. Other conditions that are not suitable for

Exclusion criteria: (1) Known or suspected allergy to the components of zedoary turmeric oil injection, or allergic constitution;
(2) Mild and severe patients.
(3) Combined with severe liver disease (such as Child-Pugh score >=C, TBIL >=2 times the upper limit of normal, ALT/AST >=5 times the upper limit of normal);
(4) Combined with severe renal insufficiency

Exclusion criteria: 1. Patients with serious primary diseases such as severe mental and psychological disorders, heart and brain vessels diseases, liver and kidney diseases, endocrine diseases, hematopoietic system diseases and malignant tumors;
2. Patients are known to be allergic to trail drugs or a certain Chinese medicine of this study;
3. Women are

Exclusion criteria: (1) Combined with high blood pressure, diabetes, coronary heart disease and other underlying disease;
(2) Combined with malignant tumor or a medical history of malignant tumor;
(3) Pregnant or lactating women;
(4) Suffering from severe mental illness;
(5) Medical history of unstable angor pectoris or myocardial?infarction within 6

Exclusion criteria: 1. Patients with cardiopulmonary resuscitation at the end of the disease;
2. Pregnant women.

Exclusion criteria: 1. There is clear evidence of bacterial infection in respiratory tract infection caused by basic diseases such as primary immunodeficiency disease, congenital respiratory tract malformation, congenital heart disease, gastroesophageal reflux disease and pulmonary dysplasia;
2. Subjects with the following conditions: asthma requiring

Exclusion criteria: (1) Chronic obstructive pulmonary disease;
(2) Cardiorenal insufficiency;
(3) Pregnant women

Exclusion criteria: (1) pregnant or breast-feeding women;
(2) ALT/AST > 5 times ULN, neutrophils < 0.5x10^9/L, platelets less than 50x10^9/L;
(3) definite diagnosis of rheumatoid immunity, malignant tumor and other related diseases;
(4) patients with long-term oral immunoregulatory drugs or who had used large doses of glucocorticoids within 14 days;
(5) allergic

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Exclusion criteria: Patients with hypersensitivity to plasma products; patients with severe transfusion reactions in the past; patients with acute pulmonary edema, congestive heart failure, pulmonary embolism, malignant hypertension, polycythemia vera, extreme renal failure and other diseases.

Exclusion Criteria:

Cases of severe vomiting that make it difficult to take the drug orally
Allergic to Favipiravir or tocilizumab
Pregnant and lactating women

Exclusion criteria: Patients with difficult airways;Patients who need to remain tracheal intubation to return to the ward after surgery;After signing the informed consent, those who were conscious and awake during the perioperative period refused to continue to participate in the test.

Exclusion Criteria:

Unable to obtain informed consent;
Patients with severe liver dysfunction (Child Pugh score ≥ C, or AST > 5 times of the upper limit), severe renal dysfunction (estimated glomerular

Exclusion criteria: 1. Participation in the study is not in accordance with the rights and interests of the patient based on PIs judgement, or any other circumstances that investigators consider inappropriate for participation;
2. With bleeding or bleeding associated with severe coagulation disorders (except for disseminated intravascular coagulation unrelated to

Exclusion criteria: 1. Participation in the study is not in accordance with the rights and interests of the patient based on PIs judgement, or any other circumstances that investigators consider inappropriate for participation;
2. Low body weight patients (females<45kg, males<57kg);
3. With bleeding or bleeding associated with severe coagulation disorders (except for

Exclusion criteria: 1. The doctor believes that the patient is not suitable to participate in this trial, including those who may not cooperate, do not comply with the requirements of the procedure, or participate in this trial may put the patient in an unsafe situation.
2. Pregnant or lactation periods women;
3. Immunoglobulin allergy;
4. Immunoglobulin A deficiency;
5.

Exclusion criteria: 1. Critical new pneumonia infection that meets one of the following conditions: respiratory failure occurs and requires mechanical ventilation; shock; combined organ failure requires ICU monitoring and treatment
2. Known allergies to Ixekizumab or antiviral drugs;
3. ALT / AST> 5 times ULN, neutrophils <0.5x10^9/L, Platelets less than

Exclusion criteria: 1. Aged < 7-year-old;
2. previous arrhythmia;
3. previous chronic respiratory failure caused by other diseases, such as heart failure, thoracic deformity, structural lung disease, etc.;
4. hemodynamic instability;
5. severe immune deficiency or recent use of immunosuppressive agents;
6. allergic patients;
7. estimated

Exclusion criteria: 1. Cases with severe vomiting and difficulty in taking drugs orally or causing difficulty in taking drugs after oral administration;
2. Pregnant and lactating women;
3. The subjects participated other clinical trials within 30 days before admission;
4. Cases with no hope of survival, hospice care only, or deep coma and no response to supportive treatment

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Exclusion criteria: 1. Patients with severe primary diseases such as heart, kidney, lung, endocrine, blood, metabolism, and gastrointestinal tract, etc., may affect participation in the trial or may affect the outcome of the study by the judgment of the researcher;
2. Patients who have a family history of mental illness or have had a mental illness;
3. Patients who are allergic

Exclusion criteria: 1. With complications of multiple organ failure;
2. Patients with malignant tumor;
3. Pregnant women;
4. HIV positive.

Exclusion criteria: (1) Death is expected within 48 hours;
(2) Basic diseases such as primary immunodeficiency disease, acquired immunodeficiency syndrome, congenital respiratory malformation , congenital heart disease, gastroesophageal reflux disease, congenital pulmonary airway malformation and more with, the exact evidence of

Exclusion criteria: 1. With contraindications of Ulinastatin Injection;
2. Pregnant or lactating women;
3. Patients taking other urinary trypsin inhibitors;
4. Not suitable to participate in the clinical trial by investigator's judgement.

Exclusion criteria: (1) Female patients during pregnancy or lactation;
(2) Patients with allergy to Exocarpium Citri Grandis preparation, patients with intolerance to the recommended dose of Exocarpium Citri Grandis were previously applied;
(3) Patients who are taking part in clinical trials of other drugs;
(4) Patients are taking other foods or drugs with antitussive

Exclusion Criteria:

Allergies, those who are known to be allergic to research drugs or drug excipients;
The patient weighs less than 40 kg;

Exclusion criteria: 1. people who are allergic to pirenidone capsules;
2. the researchers judged that the compliance of the subjects was not good;
3. those who cannot take drugs through gastrointestinal tract;
4. any significant clinical or laboratory abnormalities that may affect the safety assessment, such as: serum creatinine > 2mg/dL, or 176.8mmol sputum alt

Exclusion criteria: 1. Gestational female patients;
2. Patients with other malignant diseases such as malignant tumours, chronic liver or kidney diseases, and reaching the end stage;
3. Patients with known psychiatric disorders;
4. Patients withdrawing from other clinical trial within 4 weeks.

Exclusion criteria: 1. Gestational female patients;
2. Patients with allergic constitution, specifically with a history of allergies to these three intervention drugs;
3. Patients with other malignant diseases such as malignant tumours, chronic liver or kidney diseases, and reaching the end stage;
4. Patients with known psychiatric disorders;
5. Patients withdrawing from

Exclusion criteria: (1) Female patients who are pregnant or planning to become pregnant within 7 months;
(2) ALT is higher than 10 times the upper limit of normal value, or total bilirubin is highter than 2 times the upper limit of normal value, or the stage of decompensation of cirrhosis, or has had decompensation of cirrhosis;
(3) The number of peripheral blood

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Exclusion criteria: 1. Having one of the following infections (influenza virus, parainfluenza virus, adenovirus, respiratory syncytial virus, rhinovirus, human partial lung virus, SARS coronavirus and other known viral pneumonia);
2. Mycoplasma pneumoniae, Chlamydia pneumoniae and bacterial pneumonia;
3. There are non-infectious diseases, such as vasculitis

Exclusion criteria: The clinical classification of patients with severe novel coronavirus infection is to meet any of the following:
1) Respiratory failure occurs and requires mechanical ventilation;
2) Shock occurs;
3) Combined failure of other organs requires ICU monitoring and treatment;
2. Those who are allergic to blood products or plasma

Exclusion criteria: 1. Patients diagnosed with the critical type of COVID-19 [In accordance with the diagnostic criteria of the Notice on Printing and Distributing COVID-19 Diagnosis and Treatment Plan (Trial Implementation Version 6th) issued by the National Health and Medical Commission], and with unstable vital signs;
2. Patients with severe primary disease of liver

Exclusion criteria: (1) Known or suspected allergy to the components of the study drug;
(2) Acute infection of other viruses and bacteria has not been cured;
(3) The function of liver and kidney was abnormal (ALT/AST>=2 times of the upper limit of normal value, SCr>1.5 times of the upper limit of normal value);
(4) Participating in clinical trials of other

Exclusion criteria: 1. sNCP caused by other non-viral infections (such as trauma);
2. WHO grade III or IV pulmonary hypertension;
3. Malignant tumor in lung and other systems;
4. Lactating women or screening women with positive blood pregnancy test;
5. Idiopathic IgA deficiency;
6. Recent cardiovascular and cerebrovascular events (3

Exclusion Criteria:

Severe vomiting or difficulty ingesting medication
Woman who are pregnant or during lactation
Patients received lopinavir/ritonavir, ribavirin, interferon and monoclonal

Exclusion criteria: (1) Critical patients;
(2) TCM diagnosis of yang deficiency syndrome, manifested as limb warm, thin stools, chills, etc.;
(3) Patients who cannot guarantee compliance during treatment, and patients who are difficult to administer by oral or nasal route;
(4) Patients with severe primary respiratory disease or other pathogenic microbial

Exclusion criteria: (1) Patients who are unwilling to cooperate during the treatment;
(2) Patients with neocoronary pneumonia converted to critically ill during the treatment;
(3) Patients with mental illness who have severe primary diseases such as hematopoietic system, liver, cardiovascular, lung and kidney;
(4) Those who are allergic to alcohol

Exclusion criteria: 1. Other major mental disorders;
2. Weakness affects participation in the experiment;
3. Communication barriers;
4. Already participated in regular cognitive training;
5. Take psychotropic or other drugs that are known to affect mood;
6. History of major neurological deficits, including history of stroke, transient ischemic attack or

Exclusion criteria: (1) Those who are allergic to pirfenidone;
(2) The investigator judges that the subject's compliance is poor.
(3) Patients with severe liver disease;
(4) pregnant and lactating patients;
(5) Those who have severe kidney disease or need dialysis treatment;
(6) Those who need to take fluvoxamine at the same time

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Exclusion criteria: (1) Patients allergic to chloroquine and hydroxychloroquine;
(2) Women during pregnancy;
(3) Patients with heart, lung, kidney, brain, blood and other important organs with severe diseases accompanied by insufficiency;
(4) Patients with retinal disease, hearing loss or hearing loss;
(5) Patients with severe neurological and

Exclusion Criteria:

Other types of viral pneumonia, or bacterial pneumonia.
The clinical classification is mild, moderate or critical;
Patients with malignant blood or solid tumor

Exclusion criteria: 1. severe vomiting and difficulty in taking oral medication or in taking oral medication;
2. pregnant and nursing women;
3. subjects received lopinavir/ridonavir, ribavirin, monoclonal antibodies and other specific antiviral drugs in the week prior to admission;
4. cases of respiratory failure requiring mechanical ventilation;
5. cases of shock;

Exclusion Criteria:

Have other significant diseases other than COVID-19, that is, a disease that, according to the investigator's judgment, can cause the subject to be at risk due to participation in the study, or affect the study results and the ability of

Exclusion Criteria:

Patients participating in clinical trials of other drugs;
pregnant or lactating women;
ALT / AST> 5 times ULN, or neutrophils <0.5 * 109 / L, or platelets less than

Exclusion Criteria:

patients with mild COVID-19 pneumonia
liver dysfunction
concomitant with other active infection

Exclusion criteria: (1) Basic diseases: Patients with severe diabetes, cardiopulmonary insufficiency, chronic kidney disease, chronic liver failure, cirrhosis, etc. before onset; tumor patients; patients with azotemia;
(2) Not willing to participate in the experiment;

Exclusion criteria: 1. Combining patients with severe primary respiratory diseases, or with other pathogenic microbial pneumonia that needs to be identified in relation to COVID-19;
2. Combining systems such as heart, brain, lung, liver and kidney syllading with severe primary diseases, as well as mental illness;
3. Those who are unconscious and unable to

Exclusion Criteria:

Receive any clinical trial drug treatment for COVID-2019 within 30 days before the screening assessment;
Severe liver disease (e.g., Child Pugh score >=C or AST> 5 times of the

Exclusion criteria: 1) Previous history of chronic bronchitis, emphysema, interstitial lung disease or pulmonary heart disease;
2) Combining with other serious diseases: such as those who have suffered myocardial infarction and uncontrollable diabetes within 6 months, and are considered unsuitable to participate in the trial;
3) People with active peptic ulcer;

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Exclusion criteria: 1. In pregnancy, breast-feeding female patients;
2. to determine the history of chloroquine phosphate, rabeprazole drug allergy patients;
Patients suffering from diseases of the blood system;
3. patients suffering from chronic liver and kidney diseases and reaching the terminal stage;
4. Serious heart, lung dysfunction and mental illness;

Exclusion Criteria:

Those allergic to fapilavir;
Pregnant or lactating women;
Unstable liver, kidney, and heart diseases;

Exclusion Criteria:

Combined with severe heart, lung (diagnosed with interstitial lung disease, bronchial asthma, chronic obstructive pulmonary disease, etc.), liver and kidney disease or with endocrine, rheumatic, neurologic, malignant and other

Exclusion criteria: (1) patients without definite diagnosis of novel coronavirus pneumonia;
(2) combined with a end-stage disease with a life expectancy of less than 1 year;
(3) intolerance to cardiopulmonary monitoring recorder;
(4) regular follow-up is not possible.

Exclusion criteria: 1. Patients with primary and secondary cardiovascular, cerebrovascular, renal, endocrine, neurological and blood system diseases, tumors and tuberculosis;
2. Patients who have participated in or are participating in other clinical trials within one month;
3. Patients with mental disorders who can not express their feelings;
4. There are other

Exclusion Criteria:

Family history of seizure, epilepsy, brain or mental disease
Subject allergic to any component of the investigational vaccine, or a more severe allergic reaction and history of allergies in the past

Exclusion criteria: 1) left heart failure pulmonary edema;
2) pregnancy and lactation or perinatal period;
3) liver failure MELD score >40;
4) a history of deep vein thrombosis or pulmonary embolism in the past 3 months;
5) cases that the researcher judged to be unsuitable for the group

1. Confirmed cases, suspected cases or asymptomatic cases with COVID-19 (refer to Information System of China Disease Prevention and Control);
2. COVID-19 antibody test positive;
3. Fever (armpit temperature > 37.0 degree C), dry cough, fatigue, nasal obstruction, runny nose, sore throat, myalgia, diarrhea, shortness of breath

Exclusion criteria: 1. Dyspnea scale mMRC 0~1 or 4;
2. Resting heart rate>100bpm;
3. Adjusting dosage of drugs which affect cardiopulmonary function or heart rate (such as: trimetazidine, salbutamol, beta-blocker);
4. Combined uncontrolled chronic disease, such as uncontrolled hypertension (resting BP=160100mmHg), uncontrolled diabetes

Exclusion criteria: 1. Patients who are difficult to be administered orally or by nasal feeding, those who are allergic to the test drug and those who are intolerant to the drug;
2. The compliance of medication cannot be guaranteed during the treatment;
3. Patients with severe primary respiratory diseases, or pneumonia caused by other pathogen;
4.

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Exclusion Criteria:

Travel history / residence history of Wuhan city and surrounding areas, or other communities with case reports within 14 days;
History of contact with a SARS-CoV-2 infection (positive in nucleic acid test)

Exclusion criteria: (1) People with severe primary diseases such as heart, brain, liver, kidney and hematopoietic system;
(2) People with pulmonary malignancy;
(3) Combined with mental, intellectual or language disorders;
(4) Pregnant women, lactating women and those who have pregnancy requirement in the last 6 months;
(5) People who cannot

Exclusion criteria: (1) Patients diagnosed with COVID-19;

(2) positive nucleic acid test;

(3) positive IgM of SARS-CoV-2;

(4) positive IgG of SARS-CoV-2;

(5) pregnant or preparing pregnant women, lactating women, men

Exclusion criteria: 1. pregnancy or lactation;
2. having a history of major diseases such as disorders of liver, kidney, endocrine or hematological systems;
3. coagulation disorder;
4. explicit history of tumors;
5. having a history of malnutrition and cachexia;
6. contraindicated by HBOT, such as pneumothorax, poor pressure adjustment of the

Exclusion criteria: 1) Patients with specific contraindications of ulinastatin;
2) Female subjects had a positive urine pregnancy test or were lactating during the screening period;
3) Patients who are using Civerex and Gabexate;
4) Factors that may increase the mortality rate, which patients with severe other basic diseases such as tumor, severe malnutrition

Exclusion Criteria:

Surgery in the lung that affects lung function;
Rely on mechanical ventilation to maintain lung function;
Chronic lung diseases affecting lung function;

Exclusion criteria: 1. History of pulmonary surgery which affected lung function;
2. Relying on mechanical ventilation to maintain lung function;
3. Complications of chronic lung disease, such as COPD;
4. Complications affecting heart function;
5. Patients with severe primary diseases may affect participation in the trial or may affect the outcome of the

Exclusion criteria: 1. Because of other reasons, such as shortness of breath, hyperhidrosis, dry cough, chest distress, palpitation, etc;
2. Patients with difficulty in taking drugs orally due to basic diseases;
3. There are serious basic diseases that affect survival, including uncontrolled clinically significant heart, lung, kidney, digestion, blood disease, neuropsychiatric

Exclusion criteria: 1. Patients with difficulty in taking oral drugs due to basic diseases;
2. Accompanied by severe basic diseases affecting survival, including uncontrolled clinically significant heart, lung, kidney, digestion, blood diseases, neuropsychiatric diseases, immune diseases, metabolic diseases, malignant tumors, and severe malnutrition;
3. Allergic

Exclusion criteria: 1. Other viral pneumonia;
2. Patients who received tumor immunotherapy (such as PD-1 / L1, CTLA4, etc.) and inflammatory factor regulators such as ulinastatin in the past 1 month;
3. Patients who have used macrolides and other antibacterial drugs in the past week;
4. Patients who have undergone organ transplantation or surgery planning

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Exclusion Criteria:

Women who have recently been pregnant or breast-feeding.
Having a history of active gastrointestinal bleeding in the past 3 months.
Blood routine examination showed that the platelet count was < 30×10^9/l

Exclusion criteria: (1) Patients with fasting water due to gastrointestinal bleeding, intestinal obstruction, etc;
(2) Patients with pre-existing severe brain, heart, liver, kidney and hematopoietic system diseases;
(3) Patients who are known to be allergic to the ingredients of the intervention drug;
(4) Patients who are unable to cooperate with follow-up

Exclusion criteria: 1. Allergic to Aliskiren or Nifedipine;
2. Pregnant or breastfeeding;
3. History of Aliskiren related angioedema;
4. Severe kidney disease or other organs failure need ICU care;
5. Cardiogenic shock or KOCK utricle;
6. Participation in anther clinical studies within 3 months of enrollment

Exclusion criteria: (1) patients with severe diseases such as cardiovascular, cerebrovascular, hematopoietic, digestive system or mental illness;
(2) pregnant and lactating women;
(3) respiratory frequency> 30 times /min, showing respiratory failure;
(4) complicated with other organ failure requires treatment by respiratory intensive care unit (ICU);
(5)

Exclusion criteria: 1. Patients with difficulty in taking oral drugs due to basic diseases;

2. Accompanied by severe basic diseases affecting survival, including uncontrolled clinically significant heart, lung, kidney, digestion, blood diseases, neuropsychiatric diseases, immune diseases, metabolic diseases, malignant tumors, and severe malnutrition;

3

Exclusion criteria: (1) Pregnant women;
(2) resting heart rate <40 or >130 beats/min;
(3) unstable angina or arrhythmia;
(4) mean arterial pressure <65mmHg or >110mmHg;
(5) peak body temperature >=38 degrees C;
(6) resting blood oxygen saturation <=93%;
(7) mechanical ventilation;
(8) respiratory rate > 40 times/min or < 5

Exclusion criteria: 1. Confirmed cases, suspected cases or asymptomatic cases with COVID-19 (refer to Information System of China Disease Prevention and Control);
2. Positive in serum antibodies (IgG and IgM) screening of COVID-19;
3. Has a history of SARS virus infection (self-reported, site information);
4. Fever (armpit temperature > 37.0 degree

Exclusion criteria: 1. Patients allergic to chloroquine and hydroxychloroquine;
2. Pregnant women;
3. Patients with severe diseases of heart, lung, kidney, brain, blood and other important organs and dysfunction;
4. Patients with retinopathy, hearing loss or hearing loss;
5. Patients with severe neurological and psychiatric diseases;
6. Subjects who, in

Exclusion criteria: 1. Known or suspected allergy to the components of BFHX.
2. Acute infections, along with other viruses or bacteria.
3. Abnormal liver and kidney function tests (ALT, AST, SCr >= 1.5 times the upper limit of normal).
4. Participating in other drug clinical trials.
5. Pregnant and lactating women; or those who cannot take effective

Exclusion criteria: 1. Accompanied by severe basic diseases affecting survival, including uncontrolled clinically significant heart, kidney, digestive, blood diseases, neuropsychiatric diseases, immune diseases, metabolic diseases, malignant tumors, severe malnutrition, etc.;
2. Unstable angina pectoris or myocardial infarction in the last 1 month;
3. Allergic

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Exclusion criteria: (1) Critical type of COVID-19 according to COVID-19 diagnosis and treatment guidelines in China (7th edition);
(2) Gastrointestinal perforation;
(3) Gastrointestinal obstruction;
(4) Recent history of intestinal fistula;
(5) Disturbance of consciousness;
(6) Gastrointestinal hemorrhage;
(7) Malignant tumors;
(8)

Exclusion criteria: (1) known or suspected allergies to the components of azivudine tablets;
(2) according to the latest version of the National Health and Medical Commission Diagnostic criteria of COVID-19, ?patients with severe novel coronavirus pneumonia was confirmed;
(3) severe liver disease (TBIL >=2 times normal upper limit; ALT,AST>=5 times normal

Exclusion Criteria:

Travel / residence history of Wuhan city and surrounding areas or other communities with case reports within 14 days prior to the enrolment;

Contact with SARS-CoV-2 infected persons (positive for nucleic acid

Exclusion Criteria:

inflammatory bowel disease;
have other sources of infection;
death is anticipate within 72 hours;

Exclusion criteria: 1. Pregnant women within 3 months of pregnancy;
2. Patients with liver and kidney disease and those with liver and kidney injury
3. Patients with blood diseases (such as leukopenia, thrombocytopenia, etc.).
4. Electrocardiogram prompting patients with severely extended QT

Exclusion criteria: 1. Patients with fatigue, anorexia, abdominal distension, loose stools due to other reasons or diseases of the digestive system;
2. Patients with difficulties in taking oral drugs due to underlying diseases and other reasons;
3. It is accompanied by serious basic diseases that affect survival, including uncontrolled clinically meaningful heart, lung, kidney

Exclusion criteria: 1. Past history of psychiatric disease or be receiving psychiatric treatment;
2. Past personality disorder, intellectual disability, brain injury or brain disease;
3. With severe cardiovascular, cerebrovascular, neuromuscular and other system diseases or other diseases that injure physical activity;
4. Past history of tumor;
5

Exclusion criteria: 1. Severe pneumonia requires mechanical ventilationcritically severe cases;
2. Estimated Time of Death is less than 48 hours;
3. There is clear evidence of bacterial infection in respiratory tract infections caused by basic diseases such as primary immunodeficiency disease, acquired immunodeficiency syndrome, congenital

Exclusion Criteria:

Phase Ia:

Contraindications for vaccination

Exclusion criteria: 1. Patients with digestive disease previously;
2. Patients with mental disorders who cannot communicate normally;
3. Patients without complete medical information.

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Exclusion criteria: 1. Patients with bronchiectasis, tuberculosis, asthma, chronic obstructive pulmonary disease, lung cancer and other primary lung diseases;
2. Progressive exacerbation of dyspnea symptoms in the past 6 months;
3. In the acute exacerbation stage of pulmonary fibrosis;
4. The random blood glucose level was more than 11.1mmol/l;
5.

Exclusion criteria: 1. The physician determines that participation in the study is not in the patient's interest, or in any other case that does not allow the program to proceed safely;
2. Severe liver disease, underlying cirrhosis or a five-fold increase in Alanine transaminase Aspartate transaminase; 3. Severe impairment of Renal Function (estimated glomerular filtration rate

Exclusion criteria: (1) patients with severe diseases such as cardiovascular, cerebrovascular, hematopoietic, digestive system or mental illness;
(2) pregnant and lactating women;
(3) respiratory frequency> 30 times /min, showing respiratory failure;
(4) complicated with other organ failure requires treatment by respiratory intensive care unit (ICU);
(5)

Exclusion criteria: 1. Those who do not meet the diagnostic criteria for new coronary pneumonia;
2. Women who are pregnant or breastfeeding, who are allergic to this medicine;
3. Combining patients with severe primary diseases such as cardiovascular, liver, kidney and hematopoietic system, mental illness;
4. Contains factors that affect the observation of

Exclusion Criteria:

Excluded for novel coronavirus (SARS-CoV-2) infection

Having one of the following evidence on SARS-CoV-2 infection:

Exclusion criteria: 1. Excluded who diagnosed as pneumonia patients with novel coronavirus infection of severe type and critical type. Diagnostic criteria reference the official guideline "Pneumonia Diagnosis and Treatment Scheme for Novel Coronavirus Infection (Trial Version 6)";
2. Patients with allergic to the ingredients of bromhexine hydrochloride tablets and/or

Exclusion Criteria:

The vital signs, physical examination and laboratory test indicators of the population specified in the plan are abnormal and have clinical significance as determined by the clinician:

1. Previously diagnosed with depression and currently taking medication
2. Prior cognitive dysfunction
3. Experienced another major stressful event (e.g. divorce, bereavement) in the past year

Exclusion criteria: Subjects who meet any of the following exclusion criteria may not be enrolled:
1. Subjects who do not meet the health standards as assessed through comprehensive physical examination, mainly including:
(1) With abnormal and clinically significant vital signs (pulse rate under conscious state < 50 beats/min or > 100 beats/min, systolic blood pressure <

Exclusion Criteria:

The vital signs and physical examination results of the population specified in the plan have clinical significance as determined by the clinician;

A history of severe allergies to any component of the test vaccine, including

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Exclusion Criteria:

Phase Ib:

Contraindications for vaccination
Exclusion Criteria:

Any clinically significant chronic or acute medical condition that makes the volunteer unsuitable for participation;
A history of significant hypersensitivity, intolerance, or allergy to any drug
Exclusion Criteria:

Any clinically significant chronic or acute medical condition that makes the volunteer unsuitable for participation;
A history of significant hypersensitivity, intolerance, or allergy to any drug
Exclusion Criteria:

Having a history of severe allergy, such as severe allergic reactions, urticaria, angioedema;
Having one of the following evidence on SARS-CoV-2 infection(previous
Exclusion criteria: 1. Those who have had lung surgery that affects lung function;
2. Those who rely on mechanical ventilation to maintain lung function;
3. Combined with chronic lung and kidney disease affecting lung and kidney function;
4. Combined with diseases that affect heart function;
5. Accompanied by serious underlying diseases that affect
Exclusion criteria: 1) Health status: medical history of heart, liver, kidney, digestive tract, nervous system, respiratory system, blood and lymphatic system, immune system, mental and metabolic abnormalities, bone and other diseases with clinical significance;
2) Allergy to biological agents or any drug components:Those who had a history of allergy and were judged
Exclusion criteria :

The results of physical examination and laboratory examination before screening are abnormal and clinically significant as judged by clinicians, or systolic blood pressure >160mmHg and/or diastolic blood pressure
Exclusion Criteria:

For adult group (age ≥18 and ≤55)

Have had any acute illness, as determined by the investigator, with or without
Exclusion Criteria:

Health status: clinically significant histories of heart, liver, kidney, digestive tract, nervous system, respiratory system, blood and lymphatic system, immune system, mental, metabolic, and bone abnormalities
Exclusion Criteria:

Exclusion criteria of prime dose:

Subjects with a medical or family history of convulsions, epilepsy

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Exclusion criteria: 1. Patients with severe mental illness or suicidal tendency;
2. Patients with serious heart, brain, lung, kidney, liver and other medical diseases or tumors;
3. Patients who can not cooperate with the investigation and intervention;
4. Pregnant or lactating women.
Those who meet any of the above items shall be excluded

Exclusion criteria: 1) Subaxillary body temperature >37.0 degree C;
2) A history of close contact with patients with COVID-19 within 21 days before vaccination;
3) Those with a history of severe acute respiratory syndrome (SARS) or Middle East respiratory syndrome (MERS);
4) Women with positive blood pregnancy test, or women in pregnancy or

Exclusion criteria: 1. Patients with any following positive 2019-nCoV screening results should be excluded:
(1) Serum antibodies (IgG and IgM) of the new crown were positive;
(2) Lung CT showed new coronal imaging features;
(3) The results of RT-PCR were positive.
2. HCV antibody, HBsAg, TP antibody, HIV antibody or tuberculosis positive (lung CT

Exclusion Criteria:

Travel history / residence history of communities with case reports within 14 days;
History of contact with a SARS-CoV-2 infection (positive in nucleic acid test)

Exclusion criteria: 1. The patients who are fasting water due to gastrointestinal bleeding and intestinal obstruction;
2. Patients with severe diseases of brain, heart, liver, kidney and hemopoietic system could not cooperate with each other, the ALT of liver function was more than 1.5 times of normal upper limit;
3. Patients who are known to be allergic to

Exclusion criteria: 1. The patients who are fasting water due to gastrointestinal bleeding and intestinal obstruction;
2. Patients with severe diseases of brain, heart, liver, kidney and hemopoietic system could not cooperate with each other, the ALT of liver function was more than 1.5 times of normal upper limit;
3. Patients who are known to be allergic to

Exclusion Criteria:

Family history of seizure, epilepsy, brain or mental disease
Subject allergic to any component of the investigational vaccine, or a more severe allergic reaction and history of allergies in the past

Exclusion Criteria:

positive IgM/IgG against SARS-CoV-2 prior to randomization.
Severeor critical illness
Uncontrolled hypertension, cardiovascular/cerebrovascular diseases,lung

Exclusion criteria: 1. Patients with fasting water due to gastrointestinal bleeding, intestinal obstruction, etc.;
2. Those who cannot cooperate with the original severe brain, heart, liver, kidney and hematopoietic system diseases; those with malignant tumors and mental and neurological disorders who cannot cooperate;
3. People with allergies or allergic to the

1. Within 14 days before vaccination, subjects have been abroad and to villages/communities experienced COVID-19 epidemics, and in contact with COVID-19 cases or suspected cases. Subjects are under isolation observation, or living in the villages/communities with COVID-19 cases or suspected cases;

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Exclusion Criteria:

1) With one of the following respiratory diseases:

Subjects with asthma history, or cannot rule out asthma based on the

Exclusion criteria: Subjects meeting any of the following exclusion criteria were not included in the study
1. The subjects who did not meet the health standards in the comprehensive physical examination mainly included:
(1) Patients with abnormal vital signs and clinical significance (awake pulse < 50 beats / min or > 100 beats / min, systolic blood pressure >= 140

Exclusion criteria: 1. Patients with mental history;
2. Patients who do not use the Internet.

Exclusion criteria: 1. Within 14 days before vaccination, subjects have been abroad and to villages/communities experienced COVID-19 epidemics, and in contact with COVID-19 cases or suspected cases. Subjects are under isolation observation, or living in the villages/communities with COVID-19 cases or suspected cases;
2. Confirmed cases, suspected cases or

Exclusion Criteria:

Travel/residence history of communities with reported cases within 14 days before trial;
History of exposure to Novel Coronavirus infection (nucleic acid positive)

Exclusion criteria: 1. Subjects with axillary body temperature above 37.0 degrees C;
2. Patients with history of severe acute respiratory syndrome (SARS) or Middle East respiratory syndrome (mers);
3. Female subjects with positive urine pregnancy test, or pregnant or lactating women;
4. Subjects with acute sinusitis or acute rhinitis within 3 days before

Exclusion Criteria:

A subject who was diagnosed with severe or critical COVID-19 (as per the Diagnosis and Treatment Protocol for Novel Coronavirus Pneumonia (Trial, 8th edition), China, or Clinical Management of COVID-19, WHO);

Exclusion Criteria:

Other medical or psychiatric condition including recent (within the past year) or active suicidal ideation/behavior or laboratory abnormality that may increase the risk of study participation or, in the investigator's judgment

Exclusion Criteria:

Have a history of close contact with a confirmed case of SARS-CoV-2, an asymptomatic infection in the previous 14 days, or a travel history/residential history in a community where a case has been reported

Exclusion Criteria:

First dose exclusion criteria:

Positive serum immunoglobulin M (IgM) and IgG to the SARS-CoV-2

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Subject can be excluded by any of the following:
1.COVID-19 antibody test is positive;
2.Those with confirmed infection of SARS or MERS virus previously.
3.Those with an acute infection within 2 weeks before vaccination.
4.Those with a history of needle sickness.

Exclusion criteria: 1. Fever is suspected or diagnosed within 72 hours before enrollment (axillary temperature >=37.3 degree C/oral temperature >=37.5 degree C), or underarm body temperature >=37.3 degree C/oral temperature >=37.5 degree C on the day of enrollment;
2. Diastolic blood pressure >=100mmHg and/or systolic blood pressure >=150mmHg;
3. Those with

Exclusion Criteria:

Patients with critical COVID-19 presenting with shock, acute respiratory distress syndrome, multiple organ failure

Patients with mild COVID-19

Exclusion criteria: 1. Pneumonia caused by other viruses, except for 2019-nCov;
2. Clear bacterial infection;
3. Subjects who have used antiviral drugs and may need another antiviral treatment during the study;
4. There are serious non-infectious pulmonary diseases, including tuberculosis, pulmonary edema and pulmonary embolism;
5. Severe

Exclusion Criteria:

Any clinically significant chronic or acute medical condition that makes the volunteer unsuitable for participation.
A history of significant hypersensitivity, intolerance, or allergy to any drug

Exclusion criteria: 1. Patients with history of allergy, convulsion, epilepsy, encephalopathy and psychosis or family history;
2. Those who are allergic to any component of the vaccine, and those who have had any history of severe allergy to the vaccine (such as acute allergic reaction, urticaria, skin eczema, dyspnea, angioneurotic edema or abdominal pain);

Exclusion criteria: 1. Subjects with history of allergy, convulsion, epilepsy, encephalopathy and psychosis or family history;
2. Patients who are allergic to any component of the vaccine, and have a history of severe allergy to the vaccine (such as acute allergic reaction, urticaria, skin eczema, dyspnea, angioneurotic edema or abdominal pain);
3. Patients with

Exclusion criteria: 1. Patients with history of allergy, convulsion, epilepsy, encephalopathy and psychosis or family history;
2. Those who are allergic to any component of the vaccine, and those who have had any history of severe allergy to the vaccine (such as acute allergic reaction, urticaria, skin eczema, dyspnea, angioneurotic edema or abdominal pain);

Exclusion criteria: 1. Those who do not meet the health standards in the comprehensive physical examination mainly include:
(1) Patients with abnormal vital signs and clinical significance (awake pulse < 50 beats / min or > 100 beats / min, systolic blood pressure >= 140 mmHg or diastolic blood pressure >= 90 mmHg);
(2) Body mass index (BMI) < 18 kg / m2 or > 30

1. There is clear evidence of bacterial infection in respiratory tract infections caused by basic diseases such as primary immunodeficiency disease, acquired immunodeficiency syndrome, congenital respiratory malformations, congenital heart disease, gastroesophageal reflux disease, and abnormal lung development;

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1. There is clear evidence of bacterial infection in respiratory tract infections caused by basic diseases such as primary immunodeficiency disease, acquired immunodeficiency syndrome, congenital respiratory malformations, congenital heart disease, gastroesophageal reflux disease, and abnormal lung development;

1. Patients with new coronavirus pneumonia confirmed by pathogenic testing and clinical manifestations or signs;
2. In the opinion of the investigatorpatients with serious diseases such as malignant disease, autoimmune disease, liver and kidney disease, blood disease, neurological disease, and endocrine disease may affect patients'

Exclusion criteria: 1.Patients who have been diagnosed with Covid-19 cases and suspected cases, have a history of contact with confirmed cases and suspected cases of new coronavirus in 14 days before signing the informed consent form, or have SARS-COV-2 infection or carriers;
2.Having diseases history of cardiovascular system, respiratory system, digestive

Exclusion Criteria:

Positive serum immunoglobulin M (IgM) and immunoglobulin G (IgG) to the SARS-CoV-2.

SARS-CoV-2 nucleic acid testing positive

1) Patients who have undergone operations that affect lung function, such as lung transplantation, lung resection, lung volume reduction, etc;
2) Combined with chronic lung diseases that affect lung function, such as severe chronic obstructive pulmonary disease;
3) Patients complicated with diseases affecting cardiac function, such as

Exclusion criteria: 1. Confirmed as a confirmed case of new coronavirus pneumonia after pathogenic testing and clinical manifestations or signs;
2. Allergies, such as those with a history of allergies to two or more drugs or foods, or those who are known to be allergic to the ingredients of this drug;
3. Women who are pregnant or breastfeeding;
4. The

Exclusion criteria: 1. Patients who have difficulty in oral medication due to underlying diseases and other reasons;
2. Patients with severe primary diseases and dysfunctions such as heart, brain, breathing, digestion, endocrinology, hematopoiesis, liver and kidney, ALT and AST are 2 times higher than the upper limit of normal, or Cr>130umol/L ;
3. People with

Exclusion Criteria:

Recurring COVID-19 patients
Subjects with any unstable conditions, a history of significant hypersensitivity, or known allergy to components of the investigational agent

1. Subjects with history of convulsion, epilepsy, Guillain Barre syndrome, encephalopathy or mental illness or family history;
2. Fever (armpit temperature > 37.0 degree C) before vaccination;
3. Subjects with history of severe allergic reactions (such as anaphylactic shock, acute anaphylaxis, urticaria, skin eczema, dyspnea, angioneurotic

Exclusion Criteria:

Travel history / residence history of communities with case reports within 14 days;
History of contact with a SARS-CoV-2 infection (positive in nucleic acid test)

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Exclusion Criteria:

Have a history of close contact with a confirmed case of SARS-CoV-2, an asymptomatic infection in the previous 14 days, or a travel history/residential history in a community where a case has been reported

1. Patients who cannot guarantee medication compliance during treatment and who are difficult to administer medication through oral nasal feeding;
2. Patients with severe primary respiratory disease, or other pathogenic microbiological pneumonia that needs to be differentiated from COVID-19 or Novel Coronavirus asymptomatic infection;

Exclusion Criteria:

have a medical history or family history of convulsion, epilepsy, encephalopathy and psychosis.

be allergic to any component of the research vaccines, or used to have a

Subjects who meet any of the following criteria will be excluded:

1. Confirmed or asymptomatic COVID-19 cases;
2. Positive SARS-CoV-2 nucleic acid test;
3. Positive SARS-CoV-2 IgG or IgM test (reference to nucleic acid test and chest CT if necessary);

Subjects who meet any of the following criteria will be excluded:

1. Confirmed or asymptomatic COVID-19 cases;
2. Positive SARS-CoV-2 nucleic acid test;
3. Positive SARS-CoV-2 IgG or IgM test (reference to nucleic acid test and chest CT if necessary);

1. Those who are allergic to any component of the vaccine, and those who have history of severe allergic reactions to the vaccine, such as acute allergic reactions, urticaria, skin eczema, dyspnea, angioedema or abdominal pain;
2. Those with fever or those who suffer from acute diseases or severe chronic diseases in the acute stage of onset;

Exclusion Criteria:

Serious cardiovascular diseases, such as arrhythmia, conduction block, myocardial infarction, severe hypertension and uncontrollable medications (on-site measurement: systolic blood pressure ≥160mmHg, diastolic blood

Exclusion Criteria:

First Dose Exclusion Criteria：

Confirmed cases of SARS-CoV-2 infection, suspected cases, asymptomatic

Exclusion Criteria:

Confirmed cases, suspected cases or asymptomatic infections of SAR-CoV-2 infection (check "China Disease Prevention and Control Information System")

1. Have been diagnosed with a COVID-19 case or suspected case, had contact with a confirmed or suspected case of Novel Coronavirus within one month before signing the informed consent, or have traveled to areas with high epidemic incidence or abroad; Or novel coronavirus infection or carrier: serum anti-SARS-CoV-2 specific antibody positive, or nasopharyngeal swab

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Exclusion Criteria:

Travel history / residence history of communities with case reports within 14 days;
History of contact with a SARS-CoV-2 infection (positive in nucleic acid test)

1. Having a history of COVID-19 or a positive nucleic acid test for COVID-19;
2. Having a history or a family history of allergy, convulsion, epilepsy, brain disease, or mental illness;
3. Being allergic to any component of vaccines, or having a history of severe allergic reaction to any vaccine;

1. Having a history of COVID-19 or a positive nucleic acid test for COVID-19;
2. Having a history or a family history of allergy, convulsion, epilepsy, brain disease, or mental illness;
3. Being allergic to any component of vaccines, or having a history of severe allergic reaction to any vaccine;

1) Patients who have undergone operations that affect lung function, such as lung transplantation, lung resection, lung volume reduction, etc;
2) Dependent on mechanical ventilation to maintain lung function, such as ventilator;
3) Combined with chronic lung diseases that affect lung function, such as

Exclusion Criteria:

the patients who are diagnosed as critically ill cases or participating in other clinical trials
women who are pregnant or lactating

Exclusion Criteria:

Medical or family history of convulsions, epilepsy, encephalopathy, and psychosis disorders;
History of allergies to any ingredient of Ad5-nCoV, history of serious allergic

Exclusion Criteria:

have the medical history or family history of convulsion, epilepsy, encephalopathy and psychosis.
be allergic to any component of the research vaccines, or used to have a

1) Axillary temperature > 37.0 degree C;
2) Patients with Corona Virus Disease 2019 (COVID-19), severe acute respiratory syndrome (SARS) or Middle East respiratory syndrome (MERS).
3) Patients with history of COVID-19 vaccines or other candidate vaccines (except sequential subgroup);

1. The new coronavirus antibody test is positive, and the new coronavirus nucleic acid test is positive;
2. Have a history of SARS/MERS virus infection (self-report, on-site inquiry);
3. Have been vaccinated against other novel coronavirus vaccines;
4. Those who have a history of allergy to phenol and Mamemin;

1. The nucleic acid test turns negative twice in a row (24 hours apart);
2. Those who meet the discharge criteria and have a positive nucleic acid after discharge;
3. Severely and critically ill patients (8th edition standard);
4. Abnormal liver and kidney function (more than 2 times the normal value);

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Exclusion Criteria:

Individuals with fever >37.3°C (axillary), or any acute illness at baseline (Day 1) or within 3 days prior to randomization. Participants meeting this criterion may be rescheduled within the relevant window. Febrile participants with

Exclusion Criteria:

Those who have had surgery within 8 weeks prior to screening or who have surgery planned during the trial, and the investigator believes that such surgery may bring potential risks to the subjects

Exclusion Criteria:

History of SARS-CoV-2 infection;
Have received any COVID-19 vaccine;
Participants with abnormal fasting blood glucose or diabetes;

Exclusion Criteria:

The results of physical examination and laboratory examination before screening were abnormal and clinically significant;
Any component of the test vaccine, including aluminum preparation, has a

Exclusion Criteria:

Confirmed or asymptomatic COVID-19 cases or SARS-CoV-2 infection(had positive in SARS-CoV-2 nucleic acid test or serological test).
Had a history of traveling or residence in domestic area of high and moderate

Exclusion Criteria:

Confirmed or asymptomatic COVID-19 cases or SARS-CoV-2 infection(had positive in SARS-CoV-2 nucleic acid test or serological test).
Had a history of traveling or residence in domestic area of high pandemic

Exclusion Criteria:

Travel history / residence history of city and surrounding areas, or other communities with case reports within 14 days;
History of contact with a SARS-CoV-2 infection (positive in nucleic acid test)

Exclusion Criteria:

Within 14 days before vaccination, subjects have been abroad and to villages/communities experienced COVID-19 epidemics, and in contact with COVID-19 cases or suspected cases. Subjects are under isolation

Exclusion Criteria:

Within 14 days before vaccination, subjects have been abroad and to villages/communities experienced COVID-19 epidemics, and in contact with COVID-19 cases or suspected cases. Subjects are under isolation

Exclusion Criteria:

Travel history / residence history of communities with case reports within 14 days prior to the study;
History of contact with a SARS-CoV-2 infection (positive in nucleic acid test)

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Exclusion Criteria:

History of SARS-CoV-2 infection.
The COVID-19 antibody (IgG and IgM) screening was positive.
History of asthma, history of allergy to vaccines or vaccine components, or
Exclusion criteria for the first dose:
1. People who have been to a foreign country or an infected village/community within 14 days prior to inoculation and have been in contact with NOVEL CORONAVIRUS infection or suspected case, who are in isolation, or who have been infected or suspected case in the same
(1) Patients whose COVID-19 treatment regimen did not meet the discharge criteria;
(2) patients with severe primary heart, liver, kidney, blood or other diseases affecting their survival;
(3) Patients with mental illness, limited cognitive ability and legal disabilities;
(1) Aged < 18 years;
(2) Patients with severe primary heart, lung, liver, kidney, blood or diseases affecting their survival;
(3) Creatinine and ALT / AST exceeded the upper limit of normal value by 1.5 times;
(1) Nucleic acid test turned negative twice continuously (24 hours interval);
(2) those who meet discharge criteria and have positive nucleic acid test after discharge;
(3) those who are in critical condition and are expected to die within 24 hours;
(1) Close contacts who can not guarantee compliance of using TCM during the treatment period, or close contacts who are difficult to take medicine by oral or nasal route;
(2) Close contacts with severe primary respiratory disease;
(3) Pregnant or parturient women. Patient who have a pregnancy plan or a
Exclusion Criteria for Dose 1:
Subjects who meet any of the following exclusion criteria should be excluded from the study:
1. Individuals whose results from a complete physical examination do not meet the health standards, mainly include:
1. Individuals whose results from a complete physical examination do not meet the health standards, mainly include:
1) Abnormal vital signs with clinical significance (pulse < 50 times/min or > 100 times/min, systolic blood pressure >=140 mmHg or diastolic blood pressure >=90 mmHg on a conscious adult);
1. Complete uveitis remission: inactive disease for 3 months after discontinuing all treatments for eye disease.
2. Any known vaccine contraindications according to manufactures instructions:
(1)Individuals with a history of anaphylaxis to any component of the vaccine;
1. The subject has a history of SARS-CoV-2 infection, or has a history of contact with SARS-CoV-2 infected persons (nucleic acid test positive) or suspected infected persons within 30 days before screening, or living abroad within 30 days before screening history;
2. Anyone who is allergic to any vaccines, drugs, foods or known excipients

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Subjects who meet any of the following exclusion criteria should be excluded from the study:
1. Those who had history of COVID-19 or COVID-19 prophylactic drugs (e.g., any history of COVID-19 vaccine that has been listed or not);
2. Those who are tested positive for SARS-CoV-2 by etiological detection

1. Patients with severe cardiovascular, cerebrovascular, liver, kidney, malignant tumors, hematological diseases, and mental diseases;
2. Severe disturbance of consciousness, dementia or suffering from certain diseases affect the evaluation of curative effect, such as aphasia, visual and hearing impairment.

Exclusion Criteria:

included a physician decision that involvement in the trial was not in the patient's best interest, known allergic reaction and / or severe allergic to arbidol, blood system dysfunction (platelet count <100×10⁹/L, hemoglobin

Exclusion Criteria:

Severe type patients who comply with any of the following：

Shortness of breath, RR > 30 times/min;

1. Patients with other serious diseases such as lung diseases, cardiovascular and cerebrovascular diseases, hematopoietic diseases, autoimmune diseases, digestive system, or mental illness;
2. Pregnant or lactating women;
3. Participate in other forms of exercise during the trial

Exclusion Criteria:

have the medical history or family history of convulsion, epilepsy, encephalopathy and psychosis.
be allergic to any component of the research vaccines, or used to have a

1. Patients with severe cardiovascular, cerebrovascular, liver, kidney, malignant tumors, hematological diseases, and mental diseases;
2. Severe disturbance of consciousness, dementia or suffering from certain diseases affect the evaluation of curative effect, such as aphasia, visual and hearing impairment.

Exclusion Criteria:

have the medical history or family history of convulsion, epilepsy,encephalopathy and psychosis.
be allergic to any component of the research vaccines, or used to have a

Exclusion Criteria:

History of SARS-CoV-2 infection;
Any serious adverse reactions that are causally related to vaccination during the primary immunization of the previous inactivated SARS-CoV-2 vaccine;

Exclusion Criteria:

Having a history of COVID-19 or a positive nucleic acid test for COVID-19;
Having a history or a family history of allergy, convulsion, epilepsy, brain disease, or mental illness;

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Exclusion Criteria:

Sever Covid-19 Infection patients
SARS-Cov-2 specific antibodies (including IgM and IgG) were positive before included
Exclusion Criteria:

History of SARS-CoV-2 infection(laboratory confirmed);
Have received three and more doses of inactivated SARS-CoV-2 vaccine;
Severe adverse reactions, such as urticaria, dyspnea, and angioneurotic
Exclusion Criteria:

Suspected or confirmed fever within 72 hours before enrollment, or armpit temperature ≥37.3℃ on the day of enrollment;
Diastolic blood pressure >100mmHg and/or systolic blood pressure
1. Static heart rate > 100 beats/min; 2. Blood pressure < 90/60mmHg, > 140/90mmHg or blood pressure fluctuation more than 20mmHg, accompanied by obvious symptoms such as dizziness and headache; 3. Blood oxygen saturation <= 95%; 4. Combined with other diseases not suitable for exercise
1. Severe vomiting and difficulty in taking or ingestion of oral medication; 2. Pregnant and lactating women; 3. People with mental illness and cognitive impairment; 4. Subjects received antiviral drugs (including interferon-alpha, ribavirin, chloroquine phosphate, Abidol, remdesivir, lopinavir/ritonavir, monoclonal antibodies) in the week before admission; 5. Cases of respiratory
Exclusion criteria for first dose: 1) Within 14 days prior to vaccination, travel abroad and villages/communities affected by COVID-19, contact with novel coronavirus infected or suspected cases, persons in the quarantine period, or novel coronavirus infected or suspected cases in the same village/community; 2.) Novel coronavirus cases, suspected cases or
1. The patient's clinical data is incomplete.
2. The investigator believes that any serious concomitant systemic diseases, conditions or obstacles should be prevented from participating in this study: (1) Have a history of grade III-IV heart failure as defined by the New York Heart Association (NYHA), or a known left ventricular ejection fraction is
1.Known or suspected allergies to the components or azivudine tablets;
2.Women who are breast-feeding during pregnancy or have a family plan during the trial period and within 6 months after the end of the trial;
3.Participating in other clinical trials or using experimental drugs within 12 weeks before administration;
Those who meet any of the following criteria shall not be included in the group:
1. There were serious noninfectious pulmonary diseases, including pulmonary tumor, pulmonary edema, atelectasis, pulmonary embolism, pulmonary eosinophilic infiltration and pulmonary vasculitis;
Exclusion Criteria:

History of novel coronavirus infection confirmed or asymptomatic infected persons or positive nucleic acid test of novel Coronavirus;
SARS virus history;

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1.Medical history of COVID-19 or prior use of any medications other than 2 doses of inactivated vaccines to prevent COVID-19 (e.g., vaccination history of any other COVID-19 vaccines of other technical platform, marketed or not marketed, or vaccination history of more than 2 doses of inactivated Sinovac COVID-19 vaccine)

Exclusion Criteria:

History of contact with a SARS-CoV-2 infection (positive in nucleic acid test) within 14 days;

History of multiple system inflammatory syndrome (MIS-C);

The subjects will be excluded from the study for any of the following reasons:

1. SARS-CoV-2 PCR test is positive.
2. Those with a history of needle sickness;
3. Those with hypertension on day 0 before boost immunization (systolic blood pressure is >= 140mmHg and/or diastolic blood pressure is >=

1. Patients who have taken Xiyanping, Reduning, Tanreqing, Jinhua Qinggan, Banlangen and other antiviral Chinese medicines and/or immunomodulators, or drugs with immunomodulatory effects for external use within 1 week before being selected;

2. Currently suffering from diseases that seriously affect the immune system

Exclusion Criteria:

History of SARS-CoV-2 infection;

History of receiving COVID-19 vaccine;

History of asthma, history of allergy to the vaccine or vaccine components, or

Exclusion criteria for the first vaccination

Medical history or family history of convulsion, epilepsy, encephalopathy and psychosis.

HIV positive

Exclusion Criteria:

History of SARS-CoV-2 infection(laboratory confirmed);

Have received inactivated SARS-CoV-2 vaccine from other manufacturers other than Sinovac Life Sciences Co., Ltd, Beijing Institute of Biological

Exclusion Criteria:

History of SARS-CoV-2 infection(laboratory confirmed);

Have received three and more doses of inactivated COVID-19 vaccine;

Known allergy to vaccines or vaccine ingredients, and serious adverse

Main Exclusion Criteria:

Have oxygen saturation (SpO2) ≤93 % on room air at sea level or a ratio of arterial oxygen partial pressure (PaO2 in millimeters of mercury) to fractional inspired oxygen (FiO2) <300, respiratory rate >30 per minute, heart rate

1. Women who are in the menstrual period (day 1 to day 4 of menstrual cramps) or breastfeeding on the day of vaccination, or men whose partners plan to become pregnant within 12 months from the screening period to the full immunization period, or plan to donate sperm and eggs;

2. Those who have a history of new coronavirus vaccination;

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Exclusion Criteria:

Patients with past medical history or current clinically significant concomitant diseases;
History of malignancies within 5 years

Exclusion criteria:

Pregnancy or lactation period;
Infected with human immunodeficiency virus (HIV), hepatitis B virus (HBV) or hepatitis C virus (HCV);

Exclusion criteria:

Pregnancy or lactation period;
Infected with human immunodeficiency virus (HIV), hepatitis B virus (HBV) or hepatitis C virus (HCV);
1. other reasons caused short breath, sweating, dry cough, chest tightness, palpitations and other pulmonary heart function;
2. Patients with difficult drugs due to basic diseases;
3. with severe basic diseases that affect survival, including Uncontrolled has a clinical heart, lung, kidney, digestion, blood disease, neuroplastic disease

1) History of COVID-19, or positive results for either SARS-CoV-2 nucleic acid and antibody tests at screening (outside China only);
2) Suspected or confirmed fever (axillary temperature >=37.3 degree C/oral temperature >=37.5 degree C) within 72 hours prior to or on the day of enrollment;

The participant may not enter the trial if ANY of the following apply:
1.SARS-CoV-2 infection confirmed by RT-qPCR detection;
2.Clinical manifestations including fever, dry cough, fatigue, nasal congestion, runny nose, sore throat, myalgia, diarrhea, shortness of breath, dyspnea, etc.;

1. Medical history of COVID-19 or prior use of any medications other than 2 doses of inactivated vaccines to prevent COVID-19;
2. Abnormal laboratory test values are observed during screening period (exceed the upper or lower limit of the normal reference range by 1.2 times) and are considered clinically significant by the investigator (stage 1 subjects

Those who meet one of the following conditions will be removed: (1) patients with complications of other serious underlying diseases, such as chronic obstructive pulmonary disease, obstructive pulmonary disease, coronary heart disease, and hypertension; (2) patients with serious mental illness; (3) patients with cognitive dysfunction who are unable to understand the trial

1. SARS-CoV-2 PCR test is positive.
2. Those with an acute infection before the third vaccination.
3. Those with a history of needle sickness.
4. Those with a metal implant within 20cm around the planned injection site.
5. Those with a tattoo, keloid formation or hypertrophic scar at the planned

Exclusion Criteria:

Have the medical history or family history of convulsion, epilepsy, encephalopathy and psychosis.
Be allergic to any component of the research vaccines, or used to have a

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Exclusion Criteria:

HIV antibody positive;
HBsAg positive;
HCV antibody positive;

Exclusion Criteria:

RT-qPCR detects active SARS-Cov-2 infection;
a history of COVID-19, SARS, MERS infection (self-report, on-site inquiry);
a high-risk factor for COVID-19 infection;

Exclusion Criteria:

Presence of fever within 72 h before vaccination (axillary temperature ≥ 37.3℃), or active tuberculosis, or in the acute phase of other diseases;
A history of severe acute respiratory syndrome (SARS), Middle East

Exclusion Criteria:

Medical condition:

Known hypersensitivity to monoclonal antibody (mAb) or investigational

① Patients who are critically ill (invasive mechanical ventilation, severe heart failure or haemodynamic instability). ② Contraindications to prone positioning ((e.g. recent abdominal or thoracic surgery or trauma, facial, pelvic or spinal fractures, untreated pneumothorax, etc.). ③ Unable to cooperate due to severe mental illness, cognitive impairment, etc. ④

(1) Unable to complete the questionnaire; (2) Unable to tolerate getting out of bed.

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Count of Primary Outcomes
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Current Primary outcome measures

Clinical improvement time of 28 days after randomization
The 7-point scale
7 points: death
6 points: admission to ECMO and / or mechanical ventilation
5 points: Hospitalized for non-invasive ventilation and / or high-flow oxygen

Lower Murray lung injury score
Lower Murray lung injury score

The incidents of acute respiratory distress syndrome (ARDS) development

SOFA score

The time to 2019-nCoV RNA negativity in patients

The virological clearance rate of throat swabs, sputum, or lower respiratory tract secretions at day 7

Critically ill patients (%)

Number of participants with improvement from severe type to common type

The rate of virus inhibition

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Time to Clinical recoveryTime to Clinical Recovery (TTCR)

Time to Clinical Improvement (TTCI)

CT of lung
CT and MRI of hip

Clinical symptoms (fever, weakness, cough) recovery rate and recovery time

Clinical symptoms (fever, weakness, cough) recovery rate and recovery time

Viral nucleic acid detection
CT Scan of the Lungs
Blood routine examination
Temperature
Height
CURB-65
PSI score
Mechanical ventilation time
Length of stay in hospital

Antipyretic time
Time of virus turning negative

pulmonary function
St Georges respiratory questionnaire, SGRQ

pulmonary function
St Georges respiratory questionnaire, SGRQ
Modified Barthel Index, MBI
Incidence of adverse events

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pulmonary function
Antipyretic time
Time of virus turning negative

pulmonary function
St Georges respiratory questionnaire, SGRQ
Modified Barthel Index, MBI

Self-rating depression scale, SDS
Self-Rating Anxiety Scale, SAS
ocial support rate scale, SSRS

Rate of disease remission
Time for lung recovery

Rate of disease remission
Time for lung recovery

Rate of disease remission
Rate and time of entering the critical stage

novel coronavirus nucleic acid clearance rate

Relief of clinical symptoms and duration

Recovery time
Ratio and time for the general type to progress to heavy
Ratio and time of severe progression to critical or even death

The incidence of adverse outcome within 14 days after admission: Patients with conscious dyspnea, SpO2 = 94% or respiratory frequency = 24 times / min in the state of resting without oxygen inhalation

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Time to conversion of 2019-nCoV RNA result from RI sample

Time to viral negativity by RT-PCR
Time to clinical improvement

Time to viral negativityby RT-PCR
Time to clinical improvement: Time from start of study drug to hospital discharge or to NEWS2<2 for 24 hours.

Severe conversion rate
Oxygenation index
2019-nCoV nucleic acid test
Chest CT

The time when the nucleic acid of the novel coronavirus turns negative
T cell recovery time

PSI

Rate of comprehensive adverse outcome

Ventilation-free days

PSI

The time required for RNA Yin conversion of novel coronavirus in respiratory or blood specimens was compared between treatment groups The difference of

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Safety

Clinical symptom recovery rate

the negative conversion ratio
the proportion of general patients with advanced severe disease
confirmed rate of suspected cases
Incidence of onset at home / designated isolation population who progressed to designated isolation treatment or were diagnosed with novel coronavirus-infected pneumonia
days of onset at home / designated isolation population who progressed to designated isolation treatment or were diagnosed with novel coronavirus-
The incidence of composite adverse outcome within 14 days after admission: Defined as (one of them) SPO2<= 93% without oxygen supplementation, PaO2/FiO2 <= 300mmHg or RR <=30 breaths per minute.

Time to disease recovery

The virological clearance rate of throat swabs, sputum, or lower respiratory tract secretions at day 3
The virological clearance rate of throat swabs, sputum, or lower respiratory tract secretions at day 5
The virological clearance rate of throat swabs, sputum, or lower respiratory

Mortality in patients

disease remission
time to clinical improvement
overall rates of clinical improvement assessed at days 7, 14, and 28
time to chest CT improvement
time to virus nucleic acid negative-conversion
ECG
Chest imaging
Complications
vital signs
NEWS2 score

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the lowest SpO2 during intubation

oxygen index
max respiratory rate
lung radiography
count of lymphocyte
temperature
Length of stay
Length of severe
oxygenation index during treatment
all-cause mortality in 28 days
Peripheral blood cell count (including white blood cells, lymphocytes

Chest imaging (CT)

Chest CT
Routine blood test
liver and renal function
TCM syndrome

Validity observation index

vital signs (Body temperature, blood pressure, heart rate, breathing rate)
Respiratory symptoms and signs (Lung sounds, cough, sputum)
Etiology and laboratory testing
PaO2/SPO2
Liquid balance

the number of days between randomised grouping and clinical improvement

time to recovery.

Time to clinical recovery

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Negative conversion rate of 2019-nCoV nucleic acid
Lung inflammation absorption ratio

Negative conversion rate of COVID-19 nucleic acid
Lung inflammation absorption ratio

Rate of conversion to severe or critical illness

cure rate

Time to Clinical recovery

28-day survival
Inflammatory factor levels

Time to Clinical recovery

14 day outcome of the subjects, including: recovery, improvement, turning critical, death.
lung CT

Recovery time

Disappearance rate of fever symptoms

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Antipyretic time
Pharyngeal swab nucleic acid negative time
Blood gas analysis
Traditional Chinese medicine syndrome score

St. George's Respiratory Questionnaire

TCM symptoms efficacy

Improvement of clinical symptoms including duration of fever
Improvement of clinical symptoms including respiratory frequency
Number of participants with treatment-related adverse events evaluated with CTCAE,version 4.0

Number of patients who have progressed to suspected or confirmed within 24 days of exposure to new coronavirus

Proportion of patients with a lung injury score reduction of 1-point or more 7 days after randomization

Time of viral nucleic acid turns negative
Antipyretic time

rate of cure

Mortality rate

Pneumonia severity index
Oxygenation index (PaO2/FiO2)

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Time to clinical recovery after treatment
Rate of aggravation

Time of conversion to be negative of novel coronavirus nucleic acid

Length of admission
mortality rate

SOFA

time and rate of temperature return to normal
time and rate of improvement of respiratory symptoms and signs (lung rhones, cough, sputum, sore throat, etc.)
time and rate of improvement of diarrhea, myalgia, fatigue and other symptoms

TCM symptom score
Antifebrile time
The time and rate of transition of new coronavirus to Yin

Body temperature returns to normal time
Pulmonary inflammation resolution time (HRCT)
Mouthwash (pharyngeal swab) at the end of treatment COVID-19 RNA negative rate

Viral nucleic acid test

14 day outcome of the subjects, including: recovery, improvement, turning critical, death.
lung CT

the incidence of treatment failure in 14 days

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The improvement proportion of pulmonary fibrosis
The improvement of lung function

TTCI (Time to Clinical Improvement)

Time to Clinical Recovery, TTCR

Length of hospital stay

Number of worsening events

Lung function

Recovery time
Pneumonia psi score

Quality of life
Anxiety assessment
Depression assessment
Major symptoms improved
the rate of Coronary virus nucleic acid negative at 7 days, 14 days, 21 days, and 28 days after Ultra Short Wave Electrotherapy
systemic inflammatory response syndrome (SIRS) scores at 7 days, 14 days, 21 days, and 28 days after Ultra Short Wave Electrotherapy

Time to Clinical recovery
Butyrate in feces

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Length of hospital stay

Time to Clinical Recovery

Arterial Blood Oxygen Saturation
TTCR,Time to Clinical recovery
temperature
respiratory rate
Lung CT

Improved respiratory system function (blood oxygen saturation) recovery time

cough

Clinical recovery time
Clinical recovery time
Changes in viral load of upper and lower respiratory tract samples compared with the baseline

Main symptom relief time

lung function
ADL
6min walk

Time to Clinical recovery

cure rate

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nospital stay
Discharge rate
Site-specific hospital metastasis rate or severe conversion rate
Body temperature normalization time
Clinical symptoms disappearance rate

Recovery rate

Clinical symptoms
Blood routine
the viral load of 2019-nCOV of throat swab
TBNK cell subsets
TH1/TH2 Cytokine

Improvement of clinical symptoms (Clinical improvement is defined as a reduction of 2 points on the 6-point scale of the patient's admission status or discharge from the hospital)

viral negative-transforming time
30-day cause-specific mortality
30-day cause-adverse events
30-day all-cause mortality
co-infections

Mean clinical recovery time (hours)

Time to Clinical Recovery

MRC breathlessness scale
6MWD

COVID-19 nucleic acid detection time from positive to negative (respiratory secretion) or (3,5,7,10 days from positive to negative rate)
Lung CT observation of inflammation absorption
Proportion of cases with progressive disease
Neutrophil count
Lymphocyte count
Monocyte / macrophage count
Monocyte / macrophage function test
NK cell count

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Time of conversion to be negative of novel coronavirus nucleic acid

Proportion of COVID-19 close contacts who have developed as confirmed cases

Body temperature
TCM syndrome integral
Murray lung injury score
The transition time of novel coronavirus nucleic acid
MuLBSTA score

pneumonia severity index (PSI)
Incidence of new organ dysfunction

Time course of body temperature (fever)
Viral load over time

Clinical recovery time

Complete Blood Count
CRP
blood coagulation
D-dimer
Virological examination of pharyngeal swab

The days from positive to negative for viral nucleic acid testing

The time when the nucleic acid of the novel coronavirus turns negative
Conversion to heavy/critical type

the score of Hamilton depression scale
the score of Hamilton anxiety scale
the score of Self-Rating Depression Scale
the score of Self-Rating Anxiety Scale
the score of Athens Insomnia Scale

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The nucleic acid of the novel coronavirus is negative
CT scan of ground glass shadow disappeared

TTCI (Time to Clinical Improvement)

Chest imaging
RNA test of COVID-19
Time to remission/disappearance of primary symptoms: defined as the number of days when the three main symptoms of fever, cough, and shortness of breath are all relieved / disappeared

Time to leave ventilator on day 28 after receiving MSCs infusion

Clinical recovery time

Chest CT scan
Nucleic acid of novel coronavirus
Routine blood test
Routine urine test
Stool routine test

Ventilation-free days
mortality

Clinical index

CT scan of the lungs
Nucleic acid detection of throat secretion

CD8+ T cells numbers
CD4+ T cell numbers
NK cell numbers
Fatality rate
Clinical recovery time

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pulmonary function
Novel coronavirus pneumonic nucleic acid test

Cure rate
Mortality

Endotracheal intubation rate
mortality
Ratio of virus nucleic acid detection to negative

TCM symptom score
Becomes negative time of COVID-19
Cure / mortality rate

Change in lesion proportion (%) of full lung volume from baseline to day 28.

Size of lesion area by chest imaging
Blood oxygen saturation

pulmonary function

Routine physical examination
Vital signs: breathing, body temperature (armpit temperature)
Blood routine
C-reactive protein
Liver and kidney function test

Clinical recovery time
Pneumonia Severity Index (PSI) score

SPO2
lesions of lung CT
temperature
Blood routine
Inflammatory biomarkers

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Length of hospital stay
Discharge time of general type
Discharge time of severe type

Clinical recovery rate of day 7

Clearance rate and time of main symptoms (fever, fatigue, cough)

Rate of composite advers outcomes: SpO2, PaO2/FiO2, respiratory rate

NRS 2002 score
BMI
triceps skinfold thickness (TSF)
prealbumin
total albumin

Lung CT

Inflammation factor

The time to 2019-nCoV RNA negativity in patients

K-bld questionnaire survey
Refers to the pulse oxygen
chest CT
blood gas

During the operation, the patient's volume of local exhaled air from the mouth and nose, the patient's heart rate, respiratory rate and blood oxygen saturation

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Lung CT after 3 days
Lung CT after 7 days
Oxygenation parameters: SpO2, Partial arterial oxygen pressure (PaO2), PaO2/FiO2
Hospital stay

Status of immune function
The time of nucleic acid turns to negative
Length of stay in-hospital

Percent of subjects who have returned to room air
Percent change of subjects return to baseline oxygen requirement

Clinical symptom improvement rate: improvement rate of clinical symptoms = number of cases with clinical symptom improvement /number of enrolling cases * 100%

The percentage of patients who convert to moderate one
The rate of shock
Endotracheal intubation ratio
Time spent on the ventilator
mortality

Pharynx swabs, lower respiratory tract samples (sputum/endotracheal aspiration/alveolar lavage), and anal swabs rt-pcr of novel coronavirus nucleic acid negative conversion rate.

Temperature returns to normal for more than 3 days
Respiratory symptoms improved significantly
Pulmonary imaging showed that acute exudative lesions were significantly improved
Negative for two consecutive tests of respiratory pathogenic nucleic acid

Post-traumatic stress disorder checklist
Clinical prognostic outcome
Il-6
Health-related quality of life inventory

Psychological status
Treatment compliance
To evaluate the difference in isolation and rehabilitation of patients after discharge

PCL
PSQI
FSI

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SpO2 during intubation
the total time of intubation
Mask ventilation for SpO2 < 90%

Psychological status

fever clearance time
Effective rate of TCM symptoms

Progression rate from mild to critical/severe

Self-rating depression scale
self-rating anxiety scale
Athens Insomnia Scale

lasting time of fever
lasting time of novel coronavirus pneumonia virus nucleic acid detected by RT-PCR and negative result rate of the novel coronavirus disease nucleic acid

lasting time of fever
lasting time of novel coronavirus pneumonia virus nucleic acid detected by RT-PCR and negative result rate of the novel coronavirus disease nucleic acid

Incidence of side effects

The time of duration of COVID-19 nucleic acid RT-PCR test results of respiratory specimens (such as throat swabs) or blood specimens change to negative.

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PaO2 / FiO2 or respiratory rate (without oxygen)
Frequency of respiratory exacerbation
Observe physical signs and symptoms and record clinical recovery time
The number and range of lesions indicated by CT and X-ray of lung
Time for cough to become mild or absent

Survival rate

Time and rate of novel coronavirus become negative.

Real-time fluorescent RT-PCR detection of pharyngeal swab were used to detect the negative conversion rate of the new coronavirus nucleic acid (tested after the end of medication) and the negative conversion time(measured every other day after the start of medication, determined according to clinical needs)
Time to clinical improvement

Time to Clinical Recovery (TTCR)

Clinical therapeutic course
Pathogenic detectio
Chest CT
Laboratory indicators (blood routine, myocardial enzyme spectrum, inflammatory cytokines, etc.)
Chest imaging
Oxygenation index
Extravascular pulmonary water
pulmonary vascular permeability index, PVPI
Inflammatory factors

Recovery time
Pneumonia psi score

Pulmonary function
6 minutes walking distance
The Short Form -36 Healthy Survey
chest computerized tomography
Nucleic acid detection of novel coronavirus
TNF-alpha
IL-6
IL-10

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Tempreture
Virus nucleic acid detection

Clinical cure rate

cough
agitation

The time from randomization to clinical improvement

Time to Virus Eradication

Time to Virus Eradication

Time to clinical recovery after randomization

Lung CT

PO2/FiO2
ROX INDEX

Time to Clinical Recovery

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Medical imaging improvement rate in patients during recovery period

Shorten the duration of the disease
Antipyretic rate
Severe rate
Time of virus nucleic acid turning negative
Negative rate of viral nucleic acid
The time for the positive nucleic acid of the novel coronavirus to turn negative
Incidence of deterioration
The defervescence time
Chest CT
Primary symptom remission rate

blood gas
SOFA score

Cough Score
Expectoration score

changes in the ratio of PaO2 to FiO2 from baseline

HRCT pulmonary fibrosis score

serum albumin
siderophilin
prealbumin
lung CT scanning result

Albumin
Cellular immune indicators (total lymphocyte count (TLC) and T lymphocyte subsets (CD3, CD4))

COVID-19 nucleic acid negative conversion rate
Causal mortality
All-cause mortality

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Incidence of COVID-19 pneumonia
COVID-19 pneumonia rate of improvement

Improvement of clinical symptoms (Clinical improvement is defined as a reduction of 2 points on the 6-point scale of the patient's admission status or discharge from the hospital)

Maximal inspiratory pressure
Respiratory muscle evaluation

The change of TCM syndrome integral of deficiency of Fei-Pi-Qi-Xu Zhen
The results and changes of fatigue assessment scale at each time point

Changes of serum inflammatory factors
Patient death risk
Drug related adverse reactions and events

Time of Improvement or recovery of respiratory symptoms
Number of days virus nucleic acid shedding
Frequency of Improvement or recovery of respiratory symptoms

Inflammation absorption on Chest CT

insomnia
anxiety
depression
comfort
Pittsburgh sleep quality index
Self-rating anxiety scale
Self-rating depression scale
CD-RISC scale

HRCT score

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COVID-19 Nucleic acid

Changes of oxygenation index (PaO2/FiO2)
Side effects in the BM-MSCs treatment group

Clearance time of virus RNA

Recovery time

Clinical response
Side effects in the treatment group

Changes of oxygenation index (PaO2/FiO2) ,blood gas test

Hospital stay or death ratio of experimental group / death ratio of control group

Self-assessment of sleep quality
Efficient of sleep improvement

TTCI

FVC

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2019-ncov nucleic acid yin rate
Blood 2019-ncov nucleic acid positive
Positive 2019-ncov nucleic acid in feces
Nasopharyngeal swab positive for 2019-ncov nucleic acid

Viral nucleic acid test negative conversion rate

Changes in high-resolution computer tomography of the lung
Change in 6-minute walking distance

Exposure time in the medical cabin
Hospitalization period
Detection rate of arrhythmia

Lung CT Score

Occurrence of adverse reactions
Anti SARS-CoV-2 S IgG antibody response(ELISA)
Neutralizing antibody response to SARS-CoV-2

Monitoring of adverse events within 24 hours after infusion (including infusion related events and severe adverse reactions associated with non-primary disease)

Incidence of adverse reactions/events

Total distances of 6 minutes walk test

syndrome
nucleic acid of nCoV-19
blood count
liver and kidney function
cardiac enzymes

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Safety indexes of adverse reactions
Immunogenicity indexes of neutralizing-antibody seroconversion rates

the rate of 2019-nCoV RNA positive

SARS-CoV-2
SARS-CoV-2 IgM Antibody
SARS-CoV-2 IgG Antibody

Oxygenation index

mild cases: the critical illness rate of subjects at weeks 2
Severe or critical cases: the Changes of PaO2/FiO2 from baseline to day 1, 3, 5 and 24 hours later after the last administration
Safety data of COVID-19 patients treated with ulinastatin were collected

The proportion of responder at week 4

Six-minute Walk Test
lung CT

VAS

Visual analog score of symptoms

Virus negative time
Medication 3, 5, 7, 9, 11, 13, 15 days mouthwash (pharyngeal swab) 2019-nCOVRNA negative rate (%)

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clinical recovery time (TTCR)
the time of SARS-CoV2 overcasting

TCM syndrome curative effect

Improvement rate and time required for improvement (hours)
Clinical status decrease by two levels
Pulmonary-related mortality
All-cause mortality

Hospital stay
Modify Borg Dyspnea Score

N/A

ADL score
MMRC score

Incidence of adverse reactions/events

Detection of nucleic acid and antibody of new coronavirus

St George's respiratory questionnaire score
Fatigue assessment scale
Six minute walking experiment

Qualitative RT-PCR results of SARS-CoV-2 nucleic acid of oral / nasopharyngeal swabs, anal swabs, sputum and blood

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SARS-Cov-2 nucleic acid test

On day 7 and 14, Change (reduction) in viral load from baseline

Safety index-incidence of adverse reactions
Immunogenicity index-seroconversion rates of neutralizing antibody

Changes in diarrhea frequency and Bristol Stool Scale

Tolerance
Viral load of nCoV
Blood routine
Immunological examination
Blood liver and kidney function test

Changes in the degree of clinical symptoms such as fatigue, poor appetite, abdominal distension, and loose stools caused by decreased digestive function of the two groups of subjects

30-second sit-to-stand test score
treadmill test score
the 9-item Patient Health Questionnaire score
the 7-item Generalized Anxiety Disorder Scale score
the Revised Version of the Impact of Event Scale score

Time to disease recovery

Adverse reactions/events rate
Adverse reactions/events rate
Seroconversion rate of Neutralizing antibodies against SARS-CoV-2 Phase IIa (Day 0, 14 schedule)
Seroconversion rate of Neutralizing antibodies against SARS-CoV-2 Phase IIa

Appetite score

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Vital Capacity
Total Vital Capacity
Forced Vital Capacity
The Predicted Forced Vital Capacity
Diffusion Capacity for Carbon Monoxide

time to recovery

Modify Borg Dyspnea Score
Fatigue scale-14
Patient Health Questionnaire
Clinical Respiratory Symptom Scale

Throat Swab Nucleic Acid Test

Correlation of adverse events with the investigational product

Rate of aggravation

The number of adverse events after intramuscular injection

Psychological health level of the patient with COVID-19, assessed using the Chinese Version of Depression Anxiety and Stress Scale-21 (DASS-21) for all participants at baseline and post- intervention (1 day later)

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Neutralizing antibody positive conversion rate

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Adverse reactions/events rate
Adverse reactions/events rate
Seroconversion rate of Neutralizing antibodies against SARS-CoV-2 Phase IIb
Seroconversion rate of IgG antibodies against SARS-CoV-2 Phase Iib

Incidence of adverse events (AEs) by CTCAE v5.0
Proportion of subjects with SAEs
Proportion of subjects with infusion-related reactions
Proportion of subjects with hypersensitivity reactions

Incidence of adverse events (AEs) by CTCAE v5.0
Proportion of subjects with SAEs
Proportion of subjects with infusion-related reactions
Proportion of subjects with hypersensitivity reactions

Dose-limiting toxicity(DLT)
Maximal Tolerable Dose(MTD)

recovery rate of pulmonary function
recovery rate of renal function

Safety and tolerability of atomization (IFN- +TFF2) combined with single dose in healthy volunteers
Vital signs
Blood drug concentration

The number of adverse events after intramuscular injection
Immunogenic end point

Occurrence of solicited local reactions in the subjects (e.g., vaccination sites: pain/tenderness, erythema/redness, induration/swelling) during the 14-days after each dose of BNT162b1 or placebo.
Occurrence of solicited systematic reactions (e.g., nausea, vomiting, diarrhea, headache, fatigue, myalgia, arthralgia, chills, loss of appetite, malaise, and

Adverse Event
Serious Adverse Event

Occurrence of adverse reactions (AR) within 7 days after each dose of the recombinant SARS-CoV-2 vaccine (Sf9 cell) or placebo.

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Psychological status
Sleep status

Local and systemic adverse reactions/events within 14 days after each dose
All adverse reactions/events within 30 days after each dose
Serious adverse events (SAE) throughout the study period

Incidence of all adverse events related to vaccine (including incidence, severity, etc.)

Safety index-incidence of adverse reactions
Immunogenicity index-seroconversion rates of neutralizing antibody

Incidence of osteonecrosis of the femoral head

Incidence of osteonecrosis of the femoral head

Safety indexes of adverse reactions
Immunogencity indexes of GMT
Immunogencity indexes of neutralizing antibody

Efficacy Measurs(Time to negative conversion in viral nucleic acid test(by RT-PCR) for diagnostic samples)
Safety Measurements : 90 of participants with treatment-related adverse events as assessed byCTCAE v5.0

TCM syndrome score

Incidence of adverse reactions/events

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(VO2max)

Specific IgG antibody
Neutralizing antibody
Cellular immunity
Photo - C grade
SAS
SDS
PSQI
PSQI

The seropositive rates and level of SARS-CoV-2 neutralizing antibody
The seropositive rates of SARS-CoV-2 IgG antibody (tested by ELISA)

Immunogenicity index-geometric mean titer(GMT) of neutralizing antibody

Serum total antibodies, IgG antibodies, and neutralizing antibodies of SARS-CoV-2 one month after the last dose of vaccination (if the total antibodies against SARS-CoV-2 are positive, the positive conversion rate and antibody levels of IgG and neutralizing antibodies will be further tested)
The positive conversion rate and antibody level of SARS-CoV-2 total antibody

SARS-CoV-2 viral load

SARS-CoV-2 serum neutralizing titers - Seroconversion rates (SCR)
The geometric mean titer (GMT) of SARS-CoV-2 serum neutralizing titers at 1 month after dose 2

The proportion of adverse reactions (AR) up to Day 28 after prime and boost vaccination of the recombinant SARS-CoV-2 vaccine (CHO cell) or placebo.

Geometric mean (GMT) of specific antibody
The incidence of adverse reaction (AR)

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Adverse events
GMT of specific antibody against S protein of SARS-CoV-2
GMT of neutralizing antibody against S protein of SARS-CoV-2

SARS-CoV-2 neutralizing antibody
RBD protein binding antibody (IgG) detection.

Recovery time

Viral load
Clinical Symptoms (Fever, Cough...)
Inflammation
Pulmonary imaging
In-hospital days

Incidence of adverse events (AEs) by CTCAE v5.0
Proportion of subjects with SAEs

Serum neutralizing antibodies, IgG antibodies, and IgM antibodies of SARS-CoV-2 28 days after the last dose of vaccination

Serum neutralizing antibodies, IgG antibodies, and IgM antibodies of SARS-CoV-2 28 days after the last dose of vaccination
Serious adverse events (SAE) during the entire study period (6 months after the full vaccination
Serum neutralizing antibodies, IgG antibodies, and IgM antibodies of SARS-

Serum neutralizing antibodies, IgG antibodies, and IgM antibodies of SARS-CoV-2 28 days after the last dose of vaccination

IgG antibody
Neutralizing antibody

The primary endpoint is time to clinical improvement (censored at Day 28), defined as the time (in days) from randomization of study treatment (remdesivir or placebo) until a decline of two categories on a six-category ordinal scale of clinical status or live discharge

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Clinical symptoms (fever, cough,expectoration,chest tightness, polypnea, dyspnea) recovery rate and recovery time

The time and rate of nucleic acid turning negative during the isolation observation period

safety, tolerability, pharmacokinetic profile and immunogenicity
AE/SAE
Cmax
AUC0-inf
AUC0-t

The incidence of adverse reaction (AR)
The incidence of Adverse Events of Special Interest (AESI)

cough vas
mMRC
Borg score

Nucleic acid test positive rate of nasal and throat swabs during the isolation medical observation period

Hamilton Depression Scale

Incidence of adverse events (AEs)
Incidence of serious adverse events (SAEs)
Change from pre-dose baseline in RBC count
Change from pre-dose baseline in WBC count
Change from pre-dose baseline in Platelets count
neutralizing antibody
specific antibody
adverse events
Adverse reactions
blood biochemistry

Safety index-incidence of adverse reactions within 7 days after each dose
Immunogenicity index-seroconversion rates of neutralizing antibody against SARS-CoV-2

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The positive conversion rate of anti-SARS-CoV-2 specific neutralizing antibody (eucivirus neutralization assays)
The incidence of adverse reaction (AR)

duration of viral shedding
aggravation rate

Incidence of solicited adverse events within 7 days after vaccination.
GMT of neutralizing antibodies against live SARS-CoV-2 virus at Day 14 after the booster vaccination.

1. Positive conversion rate of serum anti-SARS-CoV-2 RBD protein antibody, and its geometric mean titer (GMT) and geometric mean fold increase (GMI)
2. Positive conversion rate of serum anti-SARS-CoV-2 neutralizing antibody, and its GMI

Safety

SARS-CoV-2 RBD specific antibody level

Incidence of Adverse Reactions (AR)
Incidence of Serious Adverse Events (SAE)

the incidence and severity of any adverse reactions/events within 30 minutes after each dose of vaccination
the incidence of abnormal blood biochemistry, blood routine, blood coagulation function and urine routine on the 4th day after each dose of vaccination
the incidence and severity of adverse reactions/events within 0-7 days after

The Geometric Mean Titer (GMT) of anti-SARS-CoV-2 neutralizing antibody
The four-fold increase rate of anti-SARS-CoV-2 neutralizing antibody

Security Index
Security Index

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Safety index-incidence of adverse reactions

Seroconversion rate
Antibody positive rate
Neutralizing antibody level

Antibody positive rate
Seroconversion rate
Neutralizing antibody level

Visual analogue scale for cough
modified Medical Research Council Dyspnea Scale (MMRC)
Borg perceived fatigue scale

changes in absolute lymphocyte counts

GMT of anti SARS-CoV-2 specific neutralizing antibody

Incidence of adverse reactions within 28 days after the booster dose.
GMT of neutralizing antibodies against live SARS-CoV-2 virus on day 14 after the booster dose.
The positive conversion rate and antibody level of COVID-19 serum IgG after second doses of 28d vaccination;
The positive conversion rate and antibody level of 28d IgA antibody in nasopharyngeal swabs after second doses of vaccination;
Levels of novel coronavirus spiking protein specific cellular immune response

adverse events;
adverse reactions;

The change of viral load;
Prognosis;

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GMT ratio of GMT-CLO-SCB-2019-010 over GMT-CLO-SCB-2019-003

Treatment-emergent Changes in Body Temperature
Treatment-emergent Changes in Respiratory Rate
Treatment-emergent Changes in Pulse Rate
Treatment-emergent Changes in Blood Pressure
Safety Laboratory Value (Haematology)

Seroconversion rate of the neutralizing antibody to SARS-CoV-2

The number of adverse events after intramuscular injection
Immunogenic end point

Incidence of solicited adverse events (AEs) after vaccination
Incidence of unsolicited AEs after vaccination

Geometric mean titer (GMT) of SARS-CoV-2 neutralising antibodies
Seroconversion rate (SCR) of SARS-CoV-2 neutralising antibodies
Geometric mean titer (GMT) of serum IgG antibodies
Seroconversion rate (SCR) of serum IgG antibodies
Geometric mean fold rise (GMFR) of SARS-CoV-2 neutralising antibodies
Immunogenicity index of GMT of the neutralizing antibody
Immunogenicity indexes of GMT of HI antibody for each of the 4 virus strains
Immunogenicity indexes of GMC of pneumococcal IgG antibody for the three serotypes (3, 19A and 19F)

Seroconversion rate of SARS-CoV-2 neutralizing antibody
Geometric mean titer of SARS-CoV-2 neutralizing antibody

Incidence of adverse reactions/events

Geometric mean titer(GMT) of neutralizing antibody to live SARS-CoV-2 in susceptible population

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Phase I clinical trial：The incidence of adverse reactions (ARs) .
Phase II clinical trial：The incidence of adverse reactions (ARs) .
Phase II clinical trial：The geometric mean titer(GMT) of specific antibody.
Phase II clinical trial：The geometric mean titer(GMT) of specific antibody.
Blood routine;
Blood biochemical;
Blood coagulation function;
Urine routines;
Urine pregnancy (for women of childbearing age only);

Immune five;
T lymphocyte subsets;
Pulmonary function (peak velocity meter measurement);

10 day rate of mild and general type conversed to severe type;

The change of viral load;

Rate of positive 2019-nCoV RNA test;

IgG antibody;
Neutralizing antibody as measured by pseudovirus neutralization assay;
Neutralizing antibody as measured by wild-type virus neutralization assay;

IgG antibody;
Neutralizing antibody;
Cellular immunity;

To compare the time to symptomatic worsening between prompt and deferred vaccination strategies for patients with uveitis;

The incidence of adverse events;
Occurrence of AE associated with vaccination;

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IgG antibody;
Neutralizing antibody;
Cellular immunity;
Cross neutralization;

fatigue scale;

Virus negative conversion rate in the first week

Recovery rate of clinical symptoms (fever, cough, expectoration, chest tightness, shortness of breath, dyspnea) and virus negative conversion rate (RT-PCR) at day 7

lung function;
ADL;
mMRC;
SF-36 scale;

Incidence of adverse reactions within 14 days after the booster dose.
GMT of neutralizing antibodies against live SARS-CoV-2 virus on day 14 after the booster dose.

fatigue scale;

Incidence of adverse reactions within 28 days after the booster dose.
GMT of neutralizing antibodies against live SARS-CoV-2 virus on day 14 after the booster dose.

Geometric mean titer(GMT) of neutralizing antibody to live SARS-CoV-2Safety index-incidence of adverse reactions

Seroconversion rate (COVAX)
Seroconversion rate (COVAX)
Neutralizing antibody level (COVAX)
Neutralizing antibody level (COVAX)
Neutralizing antibody level (COVAX)

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Clinical status at 28 days

Immunogenicity index-GMT of neutralizing antibodies (CZ02 strain)

Immunogenicity endpoint

cardio-pulmonary function;
ADL;
Lower limb function;
balanced capacity;

Time for viral nucleic acid to turn negative;

Humoral immunity (IgG, neutralizing antibody);

nucleic acid negative conversion time;
nucleic acid negative conversion rate;

time and rate of temperature return to normal;
time and rate of improvement of respiratory symptoms and signs (lung rhones, cough, sputum, sore throat, etc.);
time and rate of improvement of diarrhea, myalgia, fatigue and other symptoms;

Hyaluronic acid;
Virus negative time;

[Positive conversion rate of SARS-COV-2 neutralizing antibody]

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Neutralizing antibody as measured by wild-type virus neutralization assay;

Immunogenicity index-Seroconversion rate of the neutralizing antibody to live SARS-CoV-2
Immunogenicity index-Seroconversion rate of the neutralizing antibody to EV71

Adverse event;
GMT of binding antibody against S protein of SARS-CoV-2;
GMT of neutralizing antibody against S protein of SARS-CoV-2;

Inflammatory factors;
Immune cell genus;

Geometric mean titer(GMT) of neutralizing antibody to live SARS-CoV-2

Incidence of adverse reaction within 28 days after each dose
GMT of neutralizing antibodies against live SARS-CoV-2 virus on day 28 after the second dose.
Immunogenicity index-GMT of neutralizing antibodies
Immunogenicity index-seroconversion rate (4-fold and above increase)of neutralizing antibodies
Immunogenicity index-seropositive rate of neutralizing antibody

Immunogenicity index-GMT of neutralizing antibodies(CZ02 strain)

Number of treatment related Aes
Virologic efficacy Evaluation

IgG antibody;
Neutralizing antibody as measured by pseudovirus neutralization test;
Neutralizing antibody as measured by wild-type virus neutralization test;

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Safety: Incidence and severity of any adverse events (AEs) occurring during the clinical trial per CTCAE V5.0

The serum neutralizing antibody level of SARS-CoV-2.

The serum neutralizing antibody level of SARS-CoV-2.

Short breath
Hyperhidrosis
Chest tightness
Dry cough
Palpitation

Efficacy endpoint: Protective efficacy of symptomatic, virologically confirmed (RT-PCR positive) COVID-19 cases 28 days after full immunization.
Safety endpoint: The incidence of adverse events (AE) from the first vaccination to 30 days after full Immunization.

The incidence of effective immune reactivation on the 14th day of enrollment (Geometric mean titers (GMTs) of neutralizing antibodies produced in response to SARS-CoV-2 live virus increased 4 times or more from baseline, or changed from negative to positive from baseline)

Neutralizing antibody as measured by wild-type virus neutralization assay
Neutralizing antibody as measured by pseudovirus neutralization assay
IgG antibody

Resilience

Body Temperature
Adverse events
GMT of binding antibody against S protein of SARS-CoV-2
GMT of neutralizing antibody against S protein of SARS-CoV-2

Incidence of adverse reactions within 28 days after the booster dose.
GMT of neutralizing antibodies against live SARS-CoV-2 virus on day 28 after the booster dose in immunogenicity cohort.

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The incidence of adverse events (AE) and adverse reactions (ADR)
The incidence of grade 3-4 adverse events and adverse reactions
The incidence of serious adverse events (SAE).

Immunogenicity of Third Dose Vaccine Booster Following Two Doses of Inactivated Vaccines
Reactogenicity of Third Dose Vaccine Booster Following Two Doses of Inactivated Vaccines
Phase I: Incidence and severity of adverse reactions (ARS) from Day 0 to Day 7 days after each dose of vaccination.
Phase II: Geometric mean titers (GMT) and seroconversion rate of total IgG antibody (ELISA method) against the SARS-CoV-2 Alpha, Beta and Delta variants on Day 14 after the second dose of vaccination;
Incidence of adverse events (AEs)
Incidence of serious adverse events (SAEs)
Incidence of adverse event of special interests (AESIs)
Safety as determined by abnormality in haematology
Safety as determined by abnormality in clinical chemistry
rate of admission to ICU
Rate of tracheal intubation
change in supplemental oxygen delivery
chest CT

Quality of life

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Measure time point of outcome and Measure method of Current Primary outcome measures (If Specified)	Count of Secondary Outcomes
#BLANK	8
Lower Murray lung injury score [Time Frame: 7 days after randomization] Murray lung injury score decreased more than one point means better outcome.The Murray scoring system range from 0 to 4 according to the severity of the condition	8
The incidents of acute respiratory distress syndrome (ARDS) development [Time Frame: 14 days] The incidence rate of acute respiratory distress syndrome (ARDS) development	2
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The virological clearance rate of throat swabs, sputum, or lower respiratory tract secretions at day 7 [Time Frame: 7 days after randomization]	5
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Number of participants with improvement from severe type to common type [Time Frame: 2 weeks] Common type: Fever, respiratory tract and other symptoms, imaging examination shows pneumonia Severe type (meeting any of the following): (1) Respiratory	0
The rate of virus inhibition [Time Frame: Day 0, 2, 4, 7, 10, 14 and 21] Novel coronaviral nucleic acid is measured in nose / throat swab at each time point.	4

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Time to Clinical recovery Time to Clinical Recovery (TTCR) [Time Frame: up to 28 days] TTCR is defined as the time (in hours) from initiation of study treatment (active or placebo) until normalisation of fever, respiratory rate, and oxygen saturation, and alleviation of cough	10
Time to Clinical Improvement (TTCI) [Censored at Day 28] [Time Frame: up to 28 days] The primary endpoint is time to clinical improvement (censored at Day 28), defined as the time (in days) from randomization of study treatment (remdesivir or placebo) until a decline of two cate	10
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Rate of disease remission [Time Frame: two weeks] A: For mild patients : fever, cough and other symptoms relieved with improved lung CT B:For severe patients : fever, cough and other symptoms relieved with improved lung CT,SPO2> 93%, or PaO2/FiO2>	5
Rate of disease remission [Time Frame: two weeks] A: For mild patients : fever, cough and other symptoms relieved with improved lung CT B:For severe patients : fever, cough and other symptoms relieved with improved lung CT,SPO2> 93%, or	5
Rate of disease remission [Time Frame: day 7] For mild patients: disease remission refers to relieved symptoms with improved lung CT For severe patients: disease remission refers to relieved symptoms with improved lung CT	5
novel coronavirus nucleic acid clearance rate [on day 6 after treatment; RT-PCR]	1
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Rate of comprehensive adverse outcome [Time Frame: 14 days] The definition of comprehensive adverse outcome is as follows: SPO2≤93% without oxygen inhalation	9
Ventilation-free days [Time Frame: on the day 28 after enrollment] days without ventilation support during 28 days after patients' enrollment	8
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The virological clearance rate of throat swabs, sputum, or lower respiratory tract secretions at day 3 [Time Frame: 3 days after randomization]	2
The virological clearance rate of throat swabs, sputum, or lower respiratory tract secretions at day 5 [Time Frame: 5 days after	
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oxygen index [baseline, 1-week, 2-week, 3-week, 4-week] max respiratory rate [baseline, 1-week, 2-week, 3-week, 4-week] lung radiography count of lymphocyte [baseline, 1-week, 2-week, 3-week, 4-week] temperature [baseline, 1-week, 2-week, 3-week, 4-week]	3
Length of stay Length of severe oxygenation index during treatment[Day 1, 3, 5, 7, 10, 14, 21, 28] all-cause mortality in 28 days	1
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the number of days between randomised grouping and clinical improvement [within 28 days admission; Clinical improvement is defined as the patient's admission status of 6 grade scale score reduced by 2 points or discharged.]	6
time to recovery.[; clinical recovery was defined as sustained (48 hours) alleviation of illness based on symptom scores (fever, cough)]	8
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Improvement of clinical symptoms including duration of fever [Time Frame: Measured from day 0 through day 28]	
Evaluation of pneumonia improvement	4
Improvement of clinical symptoms including respiratory frequency [Time Frame: Measured from day 0 through day 28]	
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Time of viral nucleic acid turns negative[After the subject's condition is significantly improved, two or more new coronavirus pneumonia virus nucleic acid tests (with an interval of more than 24 hours) are performed, and the test is negative for two consecutive times. The negative time is the time when the first	5
rate of cure[untile PCR negative; PCR]	3
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Pneumonia severity index [Time Frame: From Baseline (0W) to 12 week after treatment]	
Evaluation of Pneumonia Improvement	8
Oxygenation index (PaO2/FiO2) [Time Frame: From Baseline (0W) to 12 week after treatment]	

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Time to clinical recovery after treatment [Time Frame: within 14 days from the start of medication] Defined as random to fever, respiratory rate return to normal and cough remission over 48 hours.	14
Rate of aggravation [Time Frame: within 14 days from the start	
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the incidence of treatment failure in 14 days [Time Frame: 14 days] The clinical symptoms and signs continue to deteriorate, or new pulmonary or extrapulmonary lesions appear, or the chest imaging indicates the progress, and the patient is transferred to	4

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The improvement proportion of pulmonary fibrosis [Time Frame: Week 24]	
Evaluation of pulmonary fibrosis Improvement. pulmonary fibrosis judged by HRCT score.HRCT images are divided into four grades according to the score, and a reduction of one grade	6
TTCI (Time to Clinical Improvement)[within 28 days after randomization;]	6
Time to Clinical Recovery, TTCR[within 28 days after randomization;]	7
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Quality of life[; SF-12] Anxiety assessment[; SAS] Depression assessment [; SDS] Major symptoms improved	0
the rate of Coronary virus nucleic acid negative at 7 days, 14 days, 21 days, and 28 days after Ultra Short Wave Electrotherapy[Day 7, 14, 24, 28;]	2
systemic inflammatory response syndrome (SIRS) scores at 7 days, 14 days, 21 days, and 28 days after Ultra Short Wave	
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cough[evryday; Time of disappearance of cough]	4
Clinical recovery time[The twenty-eighth days; There are at least 2 improvements in clinical status grading (from discharge 1 to death 6) of 6 indicators in clinical evaluation]	5
Clinical recovery time[The twenty-eighth days; There are at least 2 improvements in clinical status grading (from discharge 1 to	
Main symptom relief time[; Temperature/oxygen saturation/respiratory rate]	9
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Mean clinical recovery time (hours) [Time Frame: up to 28 days]	
The clinical recovery time is defined as the time from initiation of study treatment (active or placebo) until normalisation of fever, respiratory rate, and oxygen saturation, and alleviation of cough	11
Time to Clinical Recovery[Observation until discharge or turn to severe; Duration of at least 72 hours from the start of study treatment to normal body temperature (underarm temperature <= 36.9 degrees C) and cough relief (evaluated by the cough symptom score scale) Use antipyretics, and in hours)]	10
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Time course of body temperature (fever) [Time Frame: 14 days] Compare the time course of body temperature (fever) between two groups over time.	25
Viral load over time [Time Frame: 14 days]	
Clinical recovery time[The twenty-eighth days;]	6
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Chest imaging[Before and after ozone treatment;] RNA test of COVID-19[Before and after ozone treatment;] Time to remission/disappearance of primary symptoms: defined as the number of days when the three main symptoms of fever, cough, and shortness of breath are all relieved / disappeared	6
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CT scan of the lungs[Every week; Double-lung CT examination] Nucleic acid detection of throat secretion[Every day; Pharyngeal secretion virus nucleic acid detection method]	6
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Endotracheal intubation rate[Within 14 days;] mortality[Within 30 days;] Ratio of virus nucleic acid detection to negative[14day;]	3
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Change in lesion proportion (%) of full lung volume from baseline to day 28. [Time Frame: Day 28] Evaluation of Pneumonia Improvement	19
Size of lesion area by chest imaging [Time Frame: At baseline, Day 1, Week 1, Week 2, Week 4, Week 8] Evaluation of Pneumonia change Blood oxygen saturation [Time Frame: At baseline, Day 1, Week 1, Week 2, Week 4, Week 8]	8
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Routine physical examination[Before entering the group, at the end of the sample;] Vital signs: breathing, body temperature (armpit temperature)[Before entering the group, at the end of the sample;]	0
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K-BIB questionnaire survey[4 weeks;] Refers to the pulse oxygen[4 weeks;] chest CT[4 weeks;] blood gas[4 weeks;]	7
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Status of immune function[everyday in-hospital; Flow cytometry or PCR]	
The time of nucleic acid turns to negative[everyday in-hospital; PCR]	1
Length of stay in-hospital[When out of the hospital; record]	
Percent of subjects who have returned to room air [Time Frame: 7 days]	
Percent of subjects who have returned to room air	0
Percent change of subjects return to baseline oxygen requirement [Time Frame: 7 days]	
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The percentage of patients who convert to moderate one[within 14day;]	
The rate of shock	10
Endotracheal intubation ratio	
Time spent on the ventilator	
Pharynx swabs, lower respiratory tract samples (sputum/endotracheal aspiration/alveolar lavage), and anal swabs rt-pcr of novel coronavirus nucleic acid negative conversion rate.[Day 15;]	7
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Psychological status[Enrollment and two weeks after enrollment; Chinese version of the depression-anxiety-stress scale (DASS-21)]	0
Treatment compliance[Enrollment and two weeks after enrollment; The patient's degree of cooperation with the	
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Psychological status[Enrollment and two weeks after enrollment; DASS-21]	2
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Survival rate [Time Frame: 12 weeks] Death event	1
Time and rate of novel coronavirus become negative.[Everyday; Nucleic acid test reagent]	3
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Clinical therapeutic course[; Fever, respiratory rate, blood oxygen saturation turned to normal and cough relieved for at least 72 hours. Chest CT showed that the inflammation was obviously absorbed, and the nucleic acid of respiratory pathogens was negative twice in a row (the sampling interval	2
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Clinical cure rate [Time Frame: 3 months] Definition of clinical cure: The viral load of the respiratory specimen was negative for two consecutive times (the interval between the two tests was greater than or equal to one day), the lung image improved, and the body temperature returned to	6
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The time from randomization to clinical improvement [Time Frame: No more than 28 days] The time from randomization to an improvement of two points on a seven-category ordinal scale or live discharge from the hospital	0
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Lung CT[Days 7 and 14;]	2
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changes in the ratio of PaO2 to FiO2 from baseline [Time Frame: Day 10]	14
HRCT pulmonary fibrosis score[Baseline, 7th day, 14th day, discharge period, 12th week;]	0
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COVID-19 nucleic acid negative conversion rate[12 week;] Causal mortality[12 week;] All-cause mortality[12 week;]	3

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Changes of serum inflammatory factors[; ELISA] Patient death risk Drug related adverse reactions and events Time of improvement or recovery of respiratory symptoms [Time Frame: 10 days during the intervention period] Time of improvement or recovery of respiratory symptoms Number of days virus nucleic acid shedding [Time Frame: 10 days during the intervention period]	6
#BLANK	2
insomnia[before intervention; after intervention or discharged; Insomnia severity index scale] anxiety[before intervention; after intervention or discharged; Self-rating anxiety scale] depression[before intervention; after intervention or discharged;	0
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HRCT score[Base line, week 1, 2, 4 after enrollment;]	2

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Changes of oxygenation index (PaO2/FiO2) [Time Frame: At baseline, 6 hour, Day 1, Day 3,Week 1, Week 2, Week 4, Month 6.] Evaluation of pneumonia improvement	7
Side effects in the BM-MSCs treatment group [Time Frame:	
#BLANK	5
Recovery time [Time Frame: The day from the patient's admission to the Disease Progression to critical type，up to 2 weeks.] The time from the patient's admission to the Disease Progression to critical type，up to 2 weeks	17
Clinical response [Time Frame: Up to 28 days] the efficacy of NKG2D-ACE2 CAR-NK cells in treating severe and critical 2019 new coronavirus (COVID-19) pneumonia	0
Side effects in the treatment group [Time Frame: Up to 28 days]	
Changes of oxygenation index (PaO2/FiO2) ,blood gas test [Time Frame: 12 months] Improvement of pulmonary function	4
#BLANK	4
Self-assessment of sleep quality[; PSQI] Efficient of sleep improvement	5
TTCI [Time Frame: 1-28 days] Time to Clinical Improvement	9
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2019-ncov nucleic acid yin rate[; Endoscopic mucosal biopsy]	
Blood 2019-ncov nucleic acid positive Positive 2019-ncov nucleic acid in feces Nasopharyngeal swab positive for 2019-ncov nucleic acid	0
Viral nucleic acid test negative conversion rate [Time Frame: 5 months]	
Proportion of subjects who tested negative for nucleic acid from sputum or nasopharyngeal swabs for two consecutive times(sampling time at least 24 hours)	1
Changes in high-resolution computer tomograpny of the lung [Time Frame: 3 months]	
Changes in ground-glass shadows, interstitial or air nodules found on high-resolution computer tomography	4
Change in 6-minute walking distance [Time Frame: 3 months]	
#BLANK	2
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Occurrence of adverse reactions [Time Frame: 0-14 days post vaccination]	
Anti SARS-CoV-2 S IgG antibody response(ELISA) [Time Frame: 28 days post vaccination]	6
Neutralizing antibody response to SARS-CoV-2 [Time Frame:	
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Incidence of adverse reactions/events[0-7 days after each dose of vaccination;]	11
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Safety indexes of adverse reactions [Time Frame: From Day 0 to Day 28 after each dose] Incidence of adverse reactions occured from Day 0 to Day 28 after each dose	11
Immunogenicity indexes of neutralizing-antibody seroconversion	
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SARS-CoV-2[; Nucleic acid detection] SARS-CoV-2 IgM Antibody SARS-CoV-2 IgG Antibody	8
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The proportion of responder at week 4 [Time Frame: baseline, week 4] The responder is defined as the participants with ≥ 50% average VAS decrease of chest tightness and fatigue compared with baseline. The VAS ranges from 0 to 100	15
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clinical recovery time (TTCR) [Time Frame: not more than 14 days] TTCR is defined as the study treatment (oral aspirin enteric-coated tablet) began to fever, breathing rate, blood oxygen saturation recovery, and cough relieving for at least 72 hours	0
TCM syndrome curative effect[; TCM syndrome curative effect index]	5
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ADL score[discharge from the hospital;] MMRC score[discharge from the hospital;]	0
Incidence of adverse reactions/events[0-7 days after each dose of vaccination;]	7
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Safety index-incidence of adverse reactions [Time Frame: Day 0-28 after each dose vaccination] Incidence of adverse reactions after each dose vaccination	5
Immunogenicity index-seroconversion rates of neutralizing antibody [Time Frame: 28 days after the second dose Changes in diarrhea frequency and Bristol Stool Scale [Time Frame: daily, from date of randomization until the date of discharge or date of death from any cause, assessed up to 2 weeks.] Including diarrhea in times/day, Bristol Stool Scale (the minimum	7
#BLANK	0
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Adverse reactions/events rate [Time Frame: 7 days after vaccination] Occurence of adverse reactions/events after vaccination	13
Adverse reactions/events rate [Time Frame: 28 days after vaccination]	
Appetite score[On the third day of hospitalization and after 7 days intervention; Simplified Nutritional Appetite Questionnaire]	0

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Correlation of adverse events with the investigational product [Time Frame: 12 Weeks] Any adverse event, serious adverse event (SAE) occurred during the clinical study, including clinical symptoms and abnormal vital signs, abnormal laboratory tests (complete blood	4
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The number of adverse events after intramuscular injection [Time Frame: Up to one year after the last vaccination] The main observation methods of adverse reactions mainly include laboratory examination, local reactions and systemic reactions at the administration site, vital signs, blood routine	1
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Neutralizing antibody positive conversion rate [Time Frame: 30 days after inoculation] Neutralizing antibody positive conversion rate in the pre-immunization negative population 30 days after full vaccination	8

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Adverse reactions/events rate [Time Frame: 7 days after vaccination]	
Occurence of adverse reactions/events after vaccination	7
Adverse reactions/events rate [Time Frame: 28 days after vaccination]	
Incidence of adverse events (AEs) by CTCAE v5.0 [Time Frame: up to 24 weeks]	
Proportion of subjects with SAEs [Time Frame: up to 24 weeks]	1
Proportion of subjects with infusion-related reactions [Time Frame: up to 24 weeks]	
Incidence of adverse events (AEs) by CTCAE v5.0 [Time Frame: up to 24 weeks]	
Proportion of subjects with SAEs [Time Frame: up to 24 weeks]	1
Proportion of subjects with infusion-related reactions [Time Frame: up to 24 weeks]	
Dose-limiting toxicity(DLT) [Time Frame: 7 days] DLT will be assessed by DAIDS v2.1. The measurements include clinical symptoms and abnormal vital signs, abnormal laboratory tests (complete blood cell count, serum chemistry, routine urinalysis, coagulation function, etc.) and 12-lead ECGs	6
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The number of adverse events after intramuscular injection [Time Frame: 12 months after full vaccination]	
The main observation methods of adverse reactions mainly include laboratory examination, local reactions and systemic reactions at the administration site	0
Occurrence of solicited local reactions in the subjects (e.g., vaccination sites: pain/tenderness, erythema/redness, induration/swelling) during the 14-days after each dose of BNT162b1 or placebo. [Time Frame: 14 days following each dose administration]	10
Adverse Event [Time Frame: 12 weeks] safety evaluation Serious Adverse Event [Time Frame: 12 weeks] safety evaluation	0
Occurrence of adverse reactions (AR) within 7 days after each dose of the recombinant SARS-CoV-2 vaccine (Sf9 cell) or placebo. [Time Frame: 7 days after each dose.]	12
Occurrence of solicited AR in the subjects within 7 days after each dose of recombinant SARS-CoV-2 vaccine (Sf9 cell) or	

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Safety index-incidence of adverse reactions [Time Frame: Day 0-28 after each dose vaccination] Incidence of adverse reactions after each dose vaccination.	4
Immunogenicity index-seroconversion rates of neutralizing antibody [Time Frame: The 28th day after the second dose	
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Safety indexes of adverse reactions [Time Frame: within 14 days post each vaccination] Occurrence of adverse reactions post vaccination	7
Immunogencity indexes of GMT [Time Frame: Day 28 post the second vaccination]	
Efficacy measurs(Time to negative conversion in viral nucleic acid test(by RT-PCR) for diagnostic samples) [Time Frame: 0-85days]	9
Time to negative conversion in viral nucleic acid test (by RT-PCR) for diagnostic samples, negative conversion is defined as	
TCM syndrome score[; Curative effect index of TCM syndrome]	4
Incidence of adverse reactions/events[0-28 days after each dose of vaccination;]	5

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(VO2max) [Time Frame: The change from baseline in maximum oxygen consumption (VO2max) at maximum exercise load at Week 12 of treatment.] The change from baseline in maximum oxygen consumption (VO2max) at maximum exercise load at Week 12 of treatment	32
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The seropositive rates and level of SARS-CoV-2 neutralizing antibody[Day 28 post full vaccination;] The seropositive rates of SARS-CoV-2 IgG antibody (tested by ELISA)[Day 28 post full vaccination;]	5
Immunogenicity index-geometric mean titer(GMT) of neutralizing antibody [Time Frame: Day 14 after the full course vaccination] Neutralizing antibody assay will be performed using the micro-neutralization method	4
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SARS-CoV-2 viral load [Time Frame: 7 Days] Change in time-weighted SARS-CoV-2 viral load in nasopharyngeal swabs from baseline to day 7	0
SARS-CoV-2 serum neutralizing titers - Seroconversion rates (SCR) [Time Frame: 1 Month after Dose 2] SCR of SARS-CoV-2 serum neutralizing titers at 1-month after dose 2. Seroconversion is defined as ≥4-fold rise from before vaccination to 1-month post dose 2	14
The proportion of adverse reactions (AR) up to Day 28 after prime and boost vaccination of the recombinant SARS-CoV-2 vaccine (CHO cell) or placebo. [Time Frame: 28 days after first dose]	7
Geometric mean (GMT) of specific antibody [Time Frame: 1 months after immunization in each study group] Geometric mean (GMT) of specific antibody against Severe Acute Respiratory Syndrome Coronavirus 2 (SARS-CoV-2) Spike protein Receptor-binding domain(S-RBD) protein (ELISA)	9

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Adverse events GMT of specific antibody against S protein of SARS-CoV-2 [30 days after the second dose of vaccine;] GMT of neutralizing antibody against S protein of SARS-CoV-2 [30 days after the second dose of vaccine;]	0
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Recovery time [Time Frame: From date of randomization until the date of discharge, assessed up to 6 months] The discharge criteria of Diagnosis and Treatment Protocol for Novel Coronavirus Pneumonia (Trial Version 7) are applied: a) body temperature is back to normal for more than three days Viral load Clinical Symptoms (Fever, Cough...)[everyday; observation] Inflammation Pulmonary imaging In-hospital days	16 4
Incidence of adverse events (AEs) by CTCAE v5.0 [Time Frame: up to 24 weeks] Proportion of subjects with SAEs [Time Frame: up to 24 weeks]	8
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The incidence of adverse reaction (AR) [Time Frame: 0 to 7 days after each dose]	
The incidence of adverse reaction (AR)	9
The incidence of Adverse Events of Special Interest (AESI) [Time Frame: from day 0 to day 60 after last dose]	
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Incidence of adverse events (AEs) [Time Frame: Up to Day 181]	
Incidence of serious adverse events (SAEs) [Time Frame: Up to Day 181]	3
Change from pre-dose baseline in RBC count [Time Frame:	
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Safety index-incidence of adverse reactions within 7 days after each dose [Time Frame: Day 0-7 after each dose vaccination]	
Incidence of adverse reactions within 7 days after each dose	9
Immunogenicity index-seroconversion rates of neutralizing antibody against SARS-CoV-2 [Time Frame: The 28th day after	

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The positive conversion rate of anti-SARS-CoV-2 specific neutralizing antibody (eucivirus neutralization assays) [Time Frame: 30 days after full-course vaccination in each study group]	9
The incidence of adverse reaction (AR) [Time Frame: 0 to 7	
#BLANK	7
Incidence of solicited adverse events within 7 days after vaccination. [Time Frame: Within 7 days after vaccination]	
Incidence of solicited adverse events within 7 days after vaccination.	10
GMT of neutralizing antibodies against live SARS-CoV-2 virus at	
1. Positive conversion rate of serum anti-SARS-CoV-2 RBD protein antibody, and its geometric mean titer (GMT) and geometric mean fold increase (GMI)	2
2. Positive conversion rate of serum anti-SARS-CoV-2 neutralizing antibody, and its GMI [on 28 days after the last	
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SARS-CoV-2 RBD specific antibody level[baseline and 28 days after the second dose of vaccination; Chemiluminescence analysis,CLIA]	0
Incidence of Adverse Reactions (AR) [Time Frame: 0-14 days after each vaccination]	
The occurrence of Adverse Reactions (AR) in all groups	16
Incidence of Serious Adverse Events (SAE) [Time Frame:	
Within the first and final vaccination up until 12 months after the the incidence and severity of any adverse reactions/events within 30 minutes after each dose of vaccination [Time Frame: through 30 minutes after each dose]	11
Phase 1	
the incidence of abnormal blood biochemistry, blood routine	
The Geometric Mean Titer (GMT) of anti-SARS-CoV-2 neutralizing antibody [Time Frame: 14 days after the full course immunization]	7
Neutralizing antibody assay will be performed using the Microcytopathic assay	
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Safety index-incidence of adverse reactions [Time Frame: Day 0-28 after each dose] Incidence rate of adverse reactions occured from the beginning of the vaccination to 28 days after the second dose.	2
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changes in absolute lymphocyte counts [Time Frame: before and 48 hours after intervention] the changes in absolute lymphocyte counts (*10^9/L) in before and 48 hours after treatment	2
GMT of anti SARS-CoV-2 specific neutralizing antibody [Time Frame: 28 days after vaccination] GMT of anti SARS-CoV-2 specific neutralizing antibody in all groups (wild type variants test).	13
Incidence of adverse reactions within 28 days after the booster dose. [Time Frame: Within 28 days after the booster dose] Incidence of adverse reactions within 28 days after vaccination.	10
GMT of neutralizing antibodies against live SARS-CoV-2 virus	
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GMT ratio of GMT-CLO-SCB-2019-010 over GMT-CLO-SCB-2019-003 [Time Frame: Day 36]	
To demonstrate that SCB-2019 vaccine in this study of Chinese participants elicits immune response that is non-inferior to that in a randomly selected subset of participants of the CLO-SCB-	0
Treatment-emergent Changes in Body Temperature [Time Frame: up to 99 days]	
Grade of treatment-emergent changes in body temperature by Common Terminology Criteria for Adverse Event (CTCAE) scale version 5.0. The scale ranges from Grade 1 (Mild) to Grade 5	10
Seroconversion rate of the neutralizing antibody to SARS-CoV-2 [Time Frame: Day 28 after the second dose]	
Seroconversion rate of the neutralizing antibody to SARS-CoV-2 at day 28 after the second dose of inactivated COVID-19 vaccine	11
The number of adverse events after intramuscular injection [Time Frame: 12 months after full vaccination]	
The main observation methods of adverse reactions mainly include laboratory examination, local reactions and systemic reactions at the administration site	0
Incidence of solicited adverse events (AEs) after vaccination [Time Frame: 7 days after the first or second vaccination]	
Percentage of participants with solicited AEs (local, systemic) for 7 days following each primary vaccination (Days 0, 28) by intensity, relevance	6
Geometric mean titer (GMT) of SARS-CoV-2 neutralising antibodies [Time Frame: 56 days]	
Neutralizing antibody activity as detected by neutralization assay expressed as GMTs at multiple time points through Day 56	5
Immunogenicity index of GMT of the neutralizing antibody [Time Frame: Day 28 after the whole vaccination of COVID-19 vaccine]	14
GMT of the neutralizing antibody to live SARS-CoV-2 at day 28 after the whole vaccination of COVID-19 vaccine	
Geometric mean titer of SARS-CSeroconversion rate of SARS-CoV-2 neutralizing antibody [Time Frame: 28 days post full primary immunization course]	11
Geometric mean titer of SARS-CoV-2 neutralizing antibody [Time Frame: 28 days post full primary immunization course]oV-	
Incidence of adverse reactions/events [Time Frame: 0-28 days after each vaccination]	8
Geometric mean titer(GMT) of neutralizing antibody to live SARS-CoV-2 in susceptible population [Time Frame: Day 28 after the second dose]	12
GMT of the neutralizing antibody to live SARS-CoV-2 at day 28 after the second dose in susceptible population	

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Phase I clinical trial：The incidence of adverse reactions (ARs) . [Time Frame: Day 0-7 days after each vaccination.] Adverse reactions (ARs) in each dose group 0-7 days after each vaccination.	11
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Virus negative conversion rate in the first week [Time Frame: first week]	8
Recovery rate of clinical symptoms (fever, cough, expectoration, chest tightness, shortness of breath, dyspnea) and virus negative conversion rate (RT-PCR) at day 7 [Time Frame: Day 7]	23
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Incidence of adverse reactions within 14 days after the booster dose. [Time Frame: Within 14 days the booster dose]	8
Incidence of adverse reactions within 14 days after vaccination.	
GMT of neutralizing antibodies against live SARS-CoV-2 virus on day 14 after the booster dose. [Time Frame: On day 14 after	
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Incidence of adverse reactions within 28 days after the booster dose. [Time Frame: Within 28 days after the booster dose]	10
Incidence of adverse reactions within 28 days after vaccination.	
GMT of neutralizing antibodies against live SARS-CoV-2 virus on day 14 after the booster dose. [Time Frame: On day 14 after	
Geometric mean titer(GMT) of neutralizing antibody to live SARS-CoV-2 [Time Frame: 1 month after booster immunization]	3
GMT of the neutralizing antibody to live SARS-CoV-2 within 1month after booster immunization of the Inactivated SARS-CoV-	
Seroconversion rate (COVAX) [Time Frame: Results obtained 28 days after the first dose (= right before the second dose)]	1
the rate of positive seroconversion against coronavirus	
Seroconversion rate (COVAX) [Time Frame: Results obtained after the second dose of vaccination (= 56 days after the first	

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Clinical status at 28 days [Time Frame: At 28 days from inclusion]	
Clinical status at 28 days assessed using a six level ordinal scale，in this scale, 1 is the minimum score and presenting a better outcome as discharge, 2 means still in-hospital but no	4
Immunogenicity index-GMT of neutralizing antibodies（CZ02 strain） [Time Frame: Day 28 after booster vaccination] GMT of neutralizing antibodies（CZ02 strain） 28 days after booster vaccination.	6
Immunogenicity endpoint [Time Frame: 14 days after vaccination] GMT and positive rate of SARS-COV-2 neutralizing antibody 14 days after vaccination with recombinant Novel Coronavirus vaccine (CHO cells)	0
Baseline, 4 weeks, 8 weeks, 12 weeks [CPET]	
Baseline, 4 weeks, 8 weeks, 12 weeks [questionnaire survey]	
Baseline, 4 weeks, 8 weeks, 12 weeks [Lower Limb Section of the Simple Fugel-Meyer Rating Scale (FMA)]	0
Baseline, 4 weeks, 8 weeks, 12 weeks [BBS]	
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nucleic acid negative conversion time; nucleic acid negative conversion rate [3 days, 7 days, 10 days, and 14 days after vaccination injection, and 14 days and 28 days after discharge]	6
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Primary endpoint: [Time Frame: 14 days after vaccination] Positive conversion rate of SARS-COV-2 neutralizing antibody at day 14 after full inoculation of recombinant Novel Coronavirus vaccine (CHO cells).	5

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Immunogenicity index-Seroconversion rate of the neutralizing antibody to live SARS-CoV-2 [Time Frame: Day 28 after the second dose of COVID-19 vaccine]	9
Seroconversion rate of the neutralizing antibody to live SARS-CoV-2 at day 28 after the second dose of COVID-19 vaccine	
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Geometric mean titer(GMT) of neutralizing antibody to live SARS-CoV-2 [Time Frame: 28 days after the full immunization]	
Geometric mean titer(GMT) of neutralizing antibody to live SARS-CoV-2 28 days after the full immunization of each lot of COVID-19 vaccine	5
Incidence of adverse reaction within 28 days after each dose [Time Frame: within 28 days after each dose]	
Incidence of adverse reaction within 28 days after each dose.	8
GMT of neutralizing antibodies against live SARS-CoV-2 virus on day 28 after the second dose. [Time Frame: On day 28 after	
Immunogenicity index-GMT of neutralizing antibodies [Time Frame: 28 days after booster dose]	
GMT of neutralizing antibody 28 days after booster dose.	3
Immunogenicity index-seroconversion rate (4-fold and above increase)of neutralizing antibodies [Time Frame: 28 days after	
Immunogenicity index-GMT of neutralizing antibodies(CZ02 strain) [Time Frame: 14 days after the booster dose vaccination]	1
GMT of neutralizing antibodies(CZ02 strain) 14 days after the booster dose vaccination	
Number of treatment related AEs [Time Frame: 29 days after the last participant is randomized]	
Any AEs and SAEs occurring during the study	15
Virologic efficacy Evaluation [Time Frame: 7 days after the last	
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Safety: Incidence and severity of any adverse events (AEs) occurring during the clinical trial per CTCAE V5.0 [Time Frame: 85 days]	7
Incidence and severity of any adverse events (AEs) occurring during the clinical trial, including serious adverse events (SAEs)	
The serum neutralizing antibody level of SARS-CoV-2. [Time Frame: 8 weeks]	
The serum neutralizing antibody level of SARS-CoV-2 is determined by serum neutralization test, which is used to determine the serum dilution that protects 50% cells from	8
The serum neutralizing antibody level of SARS-CoV-2. [Time Frame: 8 weeks]	
The serum neutralizing antibody level of SARS-CoV-2 is determined by serum neutralization test, which is used to determine the serum dilution that protects 50% cells from	8
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Incidence of adverse reactions within 28 days after the booster dose. [Time Frame: Within 28 days the booster dose]	
Incidence of adverse reactions within 28 days after the booster dose.	11
GMT of neutralizing antibodies against live SARS-CoV-2 virus	

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The incidence of adverse events （AE） and adverse reactions （ADR） [Time Frame: 0-Day 28]	
Severity of adverse events was graded according to CTCAE5.0	2
The incidence of grade 3-4 adverse events and adverse reactions [Time Frame: 0-Day 28]	
Immunogenicity of Third Dose Vaccine Booster Following Two Doses of Inactivated Vaccines [Time Frame: Day 28]	
Observe the 28th day of the subjects' live virus neutralizing antibody geometric mean titers (GMTs) and mean geometric titers fold growth (GMFR) changes	4
Phase I: Incidence and severity of adverse reactions (ARs) from Day 0 to Day 7 days after each dose of vaccination. [Time Frame: From Day 0 to Day 7 after each dose]	15
Incidence and severity of adverse reactions occured from Day 0 to Day 7 after each dose of vaccination	
Incidence of adverse events (AEs) [Time Frame: From day1 to 6 months after administration.]	
To evaluate the safety and tolerability of a single dose of 600 mg AZD7442 administered IV to Chinese participants (including those with stable medical conditions) ≥ 18 years of age	0
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Current Secondary outcome measures [Measure time point of outcome Measure method]

The key secondary endpoint was time to clinical improvement (TTCI)
Proportion of patients in each category of the 7-point scale on days 7, 14, and 28 after randomization;
mortality on day 28
Duration of mechanical ventilation(days)
The difference of PaO2/FiO2 between two groups
Lower Sequential Organ Failure Assessment (SOFA) score
Mechanical ventilation support
The difference of PaO2/FiO2 between two groups
Lower Sequential Organ Failure Assessment (SOFA) score

The time to fever resolution rate
Time to recovery of lung injury

Duration of hospitalization
Proportion of mechanical ventilation use
Mortality
mortality on day 28
The rate of negative 2019-nCoV RNA results at day 14
the rate of aggravation
the rate of adverse events
The rate of discontinuations due to adverse events
The virological clearance rate of throat swabs, sputum, or lower respiratory tract secretions at day 3
The virological clearance rate of throat swabs, sputum, or lower respiratory tract secretions at day 5
Number of participants with treatment-related adverse events as assessed by CTCAE v5.0

Oxygenation index
PCT
Antibiotic use

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The disease prorogation-temperature
The disease prorogation-respiratory function 1
The disease prorogation-respiratory function 2
The disease prorogation-respiratory function 3

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All cause mortality
Frequency of respiratory progression
Time to defervescence (in those with fever at enrolment)
Time to cough reported as mild or absent (in those with cough at enrolment rated severe or moderate)
Clinical status
Time to Hospital Discharge OR NEWS2 (National Early Warning Score 2) of ≤ 2 maintained for 24 hours.
All cause mortality
Duration (days) of mechanical ventilation

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Single symptom disappearance rate and main symptom disappearance time
Proportion of aggravation during treatment (in line with the definition of severe or critical illness in the treatment scheme)
Rate of CT improvement
Disease recovery rate
Time and rate of coronavirus become negative
Proportion of aggravation during treatment (in line with the definition of severe or critical illness in the treatment scheme)
Rate of CT improvement
Single symptom disappearance rate and main symptom disappearance time

Hepatorenal function
Heart rate
Blood pressure

Antipyretic time
Pulmonary function
mortality and recovery rate
rate of multiple organ dysfunction
Time of virus turning negative
Pulmonary function
conversion rate of severe cases
Inflammation index
Disappearance time of gastrointestinal symptoms
Syndrome of traditional Chinese Medicine

Modified Barthel Index, MBI
Incidence of adverse events
6-minute walk test, 6MWT

6-minute walk test, 6MWT

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Disappearance time of cough
Incidence of adverse events
St Georges respiratory questionnaire, SGRQ
Modified Barthel Index, MBI
6-minute walk test, 6MWT

6-minute walk test, 6MWT

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Rate of no fever
Rate of respiratory symptom remission
Rate of lung imaging recovery
Rate of CRP,ES,Biochemical criterion (CK,ALT,Mb)recovery
Rate of undetectable viral RNA
Rate of no fever [Time Frame: two weeks]
Rate of respiratory symptom remission [Time Frame: two weeks]
Rate of lung imaging recovery [Time Frame: two weeks]
Rate of CRP,ES,Biochemical criterion(CK,ALT,Mb) recovery [Time Frame: two weeks]
Rate of undetectable viral RNA [Time Frame: two weeks]
Rate of normal tempreture
Rate of respiratory symptom remission
Rate of lung imaging recovery
Rate of laboratory indicator recovery
Rate of undetectable viral RNA

time till the SARS-CoV-2 clearance

Progression rate of 2019-nCoV pneumonia
Rate of severe pneumonia or death in people who progress to 2019-nCoV pneumonia

Improvement of TCM syndromes
Relief of clinical symptoms (fever, fatigue, gastrointestinal discomfort, etc.) and duration
Lung HRCT score improvement
Average length of hospital stay
Adverse event rate
Time from admission to clinical remission: Improvement of clinical symptom was defined as:
Patients with normal body temperature > 72 hours (without taking antipyretic drugs or hormones) and no conscious dyspnea or reduction of dyspnea
The proportion of patients with normal body temperature 14 days after admission
The proportion of patients without cough 14 days after admission

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all cause mortality
Time to temperature recovery
Time to improvement of respiratory symptom/signs
Length of hospitalization/ICU stay
Usage of oxygen therapy, mechanical ventilation;
Incidence of mechanical ventilation by day14
Incidence of ICU admission by day14
Time to treatment failure
All-cause mortality by day14, day28
The number (proportion) of subjects with viral positive by RT-PCR
Incidence of mechanical ventilation by day7, day14
Incidence of ICU admission by day7, day14
Time to treatment failure, determined as death, mechanical ventilation or ICU admission
All-cause mortality by day14, day28
Incidence of complications

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CT
X-ray
Arterial blood gas analysis
Assisted breathing use time
mortality
Time of clinical remission
Rate of no fever
Rate of no cough
Rate of no dyspnea
Rate of no need for oxygen inhalation
28-days mortality
ICU length of stay
Demand for first aid measuments
Vasopressor days
Respiratory indexes
CT
X-ray
Arterial blood gas analysis
Assisted breathing use time
mortality
Differences in mean clinical recovery time between treatment groups were compared
The proportion of subjects for the novel coronavirus RNA in respiratory or blood samples at day 7 of treatment was compared between treatment groups
RNA negative ratio of subjects for The novel coronavirus in respiratory or blood samples at day 14 of treatment was compared between treatment groups

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clinical symptom disappearance time
Time and rate of virus turning negative
Length of hospital stay
Antipyretic time
Severe conversion rate

Traditional Chinese medical syndrome scale
Body temperature

body temperature
Symptom Scale

Clinical recovery rate and time.
Rate of no fever within 14 days after admission.
Rate of no cough within 14 days after admission.
Rate of no dyspnea within 14 days after admission.
Rate of no requring supplemental oxygen within 14 days after admission
Time and rate of coronavirus become negative
Clearance rate and time of main symptoms (fever, fatigue, cough)
Exacerbation (transfer to RICU) time
Routine blood test
Biochemical Indicators

Number of participants with treatment-related adverse events as assessed by CTCAE v5.0
The critical illness rate of subjects at weeks 2

Improvement rate in patients
Incidence of shock
Incidence of multiple organ failure
Days in hospital, days in ICU
Non-invasive ventilation modes and parameters
time to discharge
mortality
deterioration rate
adverse events

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the incidence of hypoxia
the times of intubation
Jaw thrust maneuver
Length of stay in ICU
Death

blood pressure
heart rate
immunoglobin level

Others (liver and kidney function, muscle enzymes, myoglobin)

Safty
Blood biochemistry index
viral nucleic acid fo 2019-nCoV

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Safety observation index

Hemodiafiltration
Mechanical ventilation time
Application of pulmonary surfactant
Oxygen time (hours)
ICU days
28-day mortality
Hospitalization time
ICU hospitalization
Invasive mechanical ventilation during infection
ECMO duration during infection
rate of composite advers outcomes.
rate of no fever (Time Frame: 14 days)
rate of no cough
rate of no dyspnea
rate of no requring supplemental oxygen
Time to defervescence
Time to absence of 2019-nCoV
Time to normalization of inflammation markers
Time to normalization of respiratory rate
Time to normalization of cough

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Lung inflammation absorption ratio
Time for body temperature to return to normal
Time of improvement in respiratory symptoms
Time to turn critical
Days in hospital
Time for body temperature to return to normal
Time of improvement in respiratory symptoms
Proportion of severe to light
All-cause mortality
Days in hospital
Rate of disisolation
TCM symptom
Body temperature
Blood routine test
Blood biochemical test
mortality
Ventilator utilization
Hospitalization day
Time to improvement of virus infection symptoms
Length of hospitalization /Healed and discharged
Time to conversion of 2019-nCoV RNA result from RI sample
Rate of preventing mild to moderate patients from shifting to severe patients
Changes in lung inflammation absorption

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Virus negative time and negative rate
Time and rate of disappearance of main symptoms (fever, fatigue, cough)
The time when the condition becomes worse (transfer to RICU)
BRT
Blood biochemistry
vital signs
oxygenation index
Routine Blood
inflammatory biomarkers
major organ function

COVID-19 RNA negative conversion rate
Disappearance rate of main symptoms(fatigue, dyspnea)

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Chest imaging
6-minute walking distance
Pulmonary function
EQ-5D
All-cause readmission
Conversion rate of common and critical illness
Nucleic acid negative time
Antipyretic time
The number of days in hospital
Time of virus nucleic acid test negative
CD4+ and CD8+ T cell count
Rate of mortality within 28-days
Size of lesion area by thoracic imaging
Clinical manifestations and severity of suspected or confirmed patients
Body temperature
Cough severity classification
Oxygenation index
Chest CT lesion size

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Arterial Oxygen Saturation
Critical (severe or critical) conversion rate
Inflammation index (CRP)
The disappearance rate and time of cough symptoms
Clinical recovery time
time to temperature normal
breath symptoms and measure
hospitalization days
Time of improvement of clinical symptoms
Remission time of fever
Duration of mechanical ventilation
Hospitalization time
Proportion of virus positive patients
Side effects in the UC-MSCs treatment group
28-days survival
Sequential organ failure assessment
C-reactive protein
Procalcitonin

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Clinical remission rate
Dynamic changes of oxygenation index
Time to cure
rate to cure
Time to defervescence

Remission time of fever

Tempreture
PSI score

cure rate
All-cause mortality
Severity rate

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Pneumonia symptoms
oxygenation index
Routine Blood
major organ function
Coagulation index
clinical cure incidence in 14 days
the duration of virus change to negative
mortality at day 30
ICU admission rate in 30 days

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The improvement proportion of pulmonary inflammation
The improvement proportion of clinical symptom
Quality of Life-BREF (QOL-BREF)
Patient Health Questionnaire-9(PHQ-9)
Generalized anxiety disorder-7(GAD-7)
Proportion of subjects with 6-grades rating form at fixed time points (days 7, 14, 21, and 28 after randomization)
All-cause mortality of 28-days
Duration of mechanical ventilation (days) of 28-days
Duration of supplemental oxygenation (days)
All-cause mortality of 28-days
Transition time of 2019-nCoV nucleic acid detection
Frequency of respiratory progression：Defined as SPO2≤ 94% on room air or PaO2/FiO2 <300mmHg and requirement for supplemental oxygen or more advanced ventilator support
Time to defervescence (days)
Length from the day of admission to the day returning to normal body temperature, from admission to two consecutive negative results of respiratory pathogenic nucleic acid test (sampling interval of at least 1 day), and from admission to the chest CT shows significant improvement.
The proportion of cases transformed from mild to severe illness
Total duration of disease
Average length of hospital stay
Time needed for healing
Negative rate of respiratory virus nucleic acid
Time from treatment to the beginning of antipyretic
Total duration of disease
Time needed for healing
Incidence of adverse events
Number of sequelae
SF-36
Clearance rate and time of main symptoms (fever, fatigue, cough、Gastrointestinal discomfort, chest tightness)
Clearance rate of single symptom
Exacerbation (transfer to RICU) time
Chest imaging score

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time to clinical recovey
the seven-category?ordinal scores at 7 days, 14 days, 21 days, and 28 days after Ultra Short Wave Electrotherapy
Improvement of gastrointestinal symptoms
Secondary bacterial infection rate
Improvement of chest CT
All cause mortality
Time to defervescence (in those with fever at enrolment)

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Vital signs
Respiratory symptoms
mental health
immune cells
All cause mortality
Length-of-stay in hospital
Length-of-stay in ICU
Days of mechanical ventilation
Modified RAP-65 and DECAF score
Frequency of exacerbations or dyspnea
Antipyretic time
Time to cough relief
Time to relieve dyspnea
The time when the 2019-ncov RT-PCR test of upper respiratory tract samples turned negative

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fever
feeble
novel coronavirus nucleic acid detection
Lung CT
Day 7, 14, 21, 28 clinical status
Time from study entry to discharge
Frequency of serious adverse drug reactions
Dynamic changes of laboratory indexes
Dynamic changes of daily vital signs (body temperature, respiratory rate, blood pressure, heart
Time to COVID-19 RNA convert negative
Other symptoms relief rate & time
Time & Rate of inflammatory biomarkers convert normal
Time to recovery
Frequency of antipyretic use
PHQ-9
Length of hospital stay
Respiratory symptoms
immune cells
Liver and renal function test
Antifebrile time
Inflammatory absorption of chest CT
Intervals from positive to negative of Nucleic acid test
Mortality rate
conversion rate of severe patients

mortality
Ventilator utilization
Hospitalization day

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Antifebrile time
Inflammatory absorption of chest CT
Intervals from positive to negative of Nucleic acid test
Time to Clinical recovery
Mortality rate

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Main clinical manifestations subsided or significantly improved (fever, dry cough, fatigue, etc.)
ICU hospitalization days
14 and 28-day all-cause mortality

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Time to CoVID-19 RT-PCR negative in upper respiratory tract specimen
Change (reduction) in CoVID-19 viral load in upper respiratory tract specimen as assessed by area under viral load curve.
Time to defervescence (in those with fever at enrolment)
Time to cough reported as mild or absent (in those with cough at enrolment rated severe or
The incidence of common Novel coronavirus pneumonia progressing to severe
Release and quarantine time
Time when common Novel coronavirus pneumonia progress to severe
Proportion to ICU
Mechanical ventilation rate

SF-36 score

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Remission time of fever

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28-day prognosis
SOFA
Mechanical ventilation time
Length of stay in ICU
Total hospital expenses
P/F ratio over time
Sequential organ failure assessment score(SOFA score) over time
Pulmonary Severity Index (PSI)
Image examination of chest over time
Proportion of subjects who progressed to critical illness or death
Time to 2019-nCoV RT-PCR negativity in upper and lower respiratory tract specimens
clinical status
Time from study entry to discharge
Frequency of serious adverse drug reactions
Dynamic changes of laboratory indexes

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Rate of progress to severe illness
Duration of fever
C reactive protein concentration in peripheral blood
Absolute value and percentage of lymphocytes in peripheral blood
Time of pulmonary imaging remission
T cell recovery time
Remission time of fever
Improvement of chest imaging
eukocyte and lymphocyte recovery time
Frequency of serious adverse drug reactions

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All-cause mortality
Time for body temperature to return to normal
Time of improvement in respiratory symptoms
Proportion of severe to light
Days in hospital
Conversion of severe cases: defined as the number of severe and critical cases that occurred after the study was included and the intervention was initiated
Incidence of medical complications during hospitalization: defined as medical complications during pneumonia isolation hospitalization due to novel coronavirus infection, including combined bacterial infections, exacerbation of underlying diseases, etc

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fever

Liver function test
renal function

ICU length of stay
Chest CT
Routine blood test
C-reactive protein
Liver and renal function

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Body temperature
3CL Mpro of Coronavirus
Routine blood test
Routine urine test
Liver function test
Arterial Oxygen Saturation
cytokines/chemokines
Hospital stay duration
Inflammation index (CRP)

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pulmonary CT
pulmonary CT
chest radiography

Length of stay

Inflammatory markers
Clinical symptoms
Liver and kidney function

Major symptom relief time
Blood gas analysis
Indicators of inflammation (CRP, ESR)
Chest CT
Change in lesion proportion (%) of full lung volume from baseline to day 10 and 90
Change in consolidation lesion proportion (%) of full lung volume from baseline to day 10, 28 and 90.
Change in ground-glass lesion proportion (%) of full lung volume from baseline to day 10, 28 and 90
Rate of mortality within 28-days
Sequential organ failure assessment
Side effects in the UC-MSCs treatment group
Electrocardiogram, the changes of ST-T interval mostly
Concentration of C-reactive protein C-reactive protein, immunoglobulin
6-minute walk test
basic ADL
IADL
anxiety
depression

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hepatic and renal function

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Conversion rate of severe type to general type
Conversion rate of general type to light type
Conversion rate of general type to severe type
Conversion rate of severe type to critical type
All-cause mortality during the trial
The rate of respiratory failure during the trial
The time from randomization to fever reduction
The time from randomization to cough relief
The time from randomization to dyspnea relief
Chest imaging
Refers to oxygen saturation
Respiratory rate
The rate and time at which the normal type progresses to the heavy type
Rate and time of progression from heavy to critical type and even death

novel coronavirus nucleic acid detection

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Nucleic acid detection of pharyngeal test
Leukocytes and lymphocytes in blood routine

7 day mortality
duration of mechanical ventilation
PD-1 expression of T cell
Absolute Lymphoblastic Counter
28 day mortality
The 7-point scale
The rate of critcal aggravation
Blood routine test outcomes
Blood biochemical test outcomes
Within 4 weeks of death from respiratory causes
Within 4 weeks of disease progression or death
Routine lymphocyte recovery
Changes in inflammatory markers of blood (il-8 et al.)
Absolute change in viral nucleic acid from baseline

New coronavirus infection before and after treatment, and new coronavirus infection among relevant medical staff (including chest CT, viral nucleic acid detection, clinical diagnosis, etc.)

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Sputum suction amount
Circulation indicators: blood pressure, heart rate

The incidence rate of complication

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The fading and obviously improving time of clinical symptoms (fever, dry cough, fatigue, etc)
The negative conversion rate of SARS-CoV-2 nucleic acid
Negative conversion time of SARS-CoV-2 nucleic acid
Abatement rate of fever transfusion .The definition of abatement rate refers to the case percentage with ≤ 37.5℃ axillary temperature and the maintenance of at least 48h
CRP
IL-6
TNF-α
Time of improvement of clinical symptoms (fever, cough)
Adverse drug reactions
1. 28-day clinical improvement time: the number of days from randomization to clinical improvement.Clinical improvement was defined as a reduction of 2 points on the 6-point scale of the patient's admission status or discharge.
2. Hospitalization duration of 28 days after admission: The number of days from randomization to discharge. Discharge criteria: body temperature returned to normal for more than 3 days;
Proportion of severe cases
Length of stay
Oxygen saturation
Mechanical ventilation time

Physical activity scale
Physical activity scale for the elderly,DISTRESS THEROMERTER

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the incidence of hypoxia
the times of intubation
7-day mortality

Treatment compliance
To evaluate the difference in isolation and rehabilitation of patients after discharge

body tempture
lung CT scan
clearance time of COVID-19
Oxygen saturation
duration of hospital stay
NEW Score
Hospital Stay
30-day all-cause mortality
Levels of Inflammatory Factors and Oxidative Stress Markersive Stress Markers

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Comparison of TCM symptom score (degree of fever, cough, sputum, chest tightness, shortness of breath, anorexia, abdominal distension, the tongue proper and the tongue coating, pulse condition)
comparison of changes in SpO2, oxygen saturation, lymphocyte count
Clinical recovery time: the body temperature returned to normal and lasting for more than 3
Comparison of TCM symptom score (degree of fever, cough, sputum, chest tightness, shortness of breath, anorexia, abdominal distension, the tongue proper and the tongue coating, pulse condition)
Comparison of changes in inflammation indicators (white blood cell count, C-reactive protein, ESR, procalcitonin) before and after treatment

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Time for improvement of clinical symptoms (eg fever, cough, dyspnea, etc.)
Imaging test improvement time (chest CT scan)
Occurrence of complications during hospitalization (such as severe sepsis, septic shock, and acute renal failure, etc.)
Mechanical ventilation time (days)

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body temperature

Level of IL2, IL4, IL6, IL10, TNF-alpha and IFN-gama
Routine blood test
lymphocyte subsets
Time to Clinical recovery
Proportion of cases with disease progression
Cough disappearance rate and time
All-cause mortality
Changes of cytokines (IL-2, IL-4, IL-6, IL-10, IL-17, INF-gama, TNF-alpha)
Time for temperature to return to normal
Time to 2019-nCoV RT-PCR negative in upper respiratory tract specimen
The rate of progression from mild/moderate type to heavy/critical type
4. Laboratory tests(white blood cell and lymphocyte count, erythrocyte sedimentation rate, C-reactive protein)

All-cause death
Adverse events

Mechanical ventilation time (days)
Total hospitalization time (days)
28-day all-cause mortality

Clearance rate and time of main symptoms (fever, fatigue, cough)
Disappearance rate of single symptom in traditional Chinese Medicine
Exacerbation (transfer to RICU) time
Chest imaging score

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Laboratory examination
Length of admission
Mortality rate
Incidence of adverse events in blood transfusion
Viral nucleic acid test negative conversion rate and days from positive to negative
Duration of fever
Lung imaging improvement time
Mortality rate because of Corona Virus Disease 2019
Rate of non-invasive or invasive mechanical ventilation when respiratory failure occurs

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The incidence of mild or common novel coronavirus pneumonia progressing to severe
Time for the main clinical manifestations to subside (fever, cough, respiratory rate, SPO2)

The incidence of mild or common novel coronavirus pneumonia progressing to severe
Time for the main clinical manifestations to subside (fever, cough, respiratory rate, SPO2)

28-day mortality
Hospitalization time
Incidence of breathing exacerbations
Time for conscious cough relief during infection (cough present when enrolled)
Time to remission of conscious dyspnea during infection (existed dyspnea upon enrollment)

Lung function
Arterial blood gas analysis

WBC
PBML
Time of Support time

All cause mortality
Frequency of exacerbation of dyspnea
Remission time of non-primary symptoms
Time for 2019-nCoV nucletic acid turning negtive in throat swabs
Duration of oxygen inhalation or noninvasive ventilation

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traditional Chinese medical syndrome scale
TCM symptom integral
TCM Constitution
body temperature
Serum antibody

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Haematological
C-reactive protein
Blood gas analysis
Urine test
Blood lipid
28 natural survival rate
TTCI (Time to Clinical Improvement)
Days in hospital
Duration of non-invasive / invasive ventilation
Proportion of severe to light

Hospital Stay
Inflammatory Factor Level

PaO2
blood oxygen saturation (SpO2)
clinical status rating on the 7-point ordinal scale
Time to Clinical Improvement (TTCI)
Duration (hours) of non-invasive mechanical ventilation or high-flow nasal catheter oxygen

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Length of hospital stay
Length from the day of admission to the day returning to normal body temperature, from admission to two consecutive negative results of respiratory pathogenic nucleic acid test (sampling interval of at least 1 day), and from admission to the chest CT shows significant improvement
Recurrence rate of COVID-19.
Height
Weight
body mass index
upper arm circumference

Causal mortality
COVID-19 nucleic acid negative conversion rate
All-cause mortality

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Improving time of main clinical symptoms(wheezing, cough, sputum, etc)
ICU hospitalization days
14 and 28-day all-cause mortality
Dyspnea evaluation
6-Minute Walking Test
Activity of Daily Living
Evluation of International Classification of Functioning, Disability and Health
Hamilton Anxiety Scale

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Duration of fever
Frequencies of progression to severe illness
Time of improvement of pulmonary imaging
Peripheral blood c-reactive protein concentration
Absolute value of peripheral blood lymphocytes

Total hospitalized time
side effects associated with interventions

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Lung function
Oxygenation index

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Clinical outcome
Hospital stay
CT Scan
Changes in viral load
Changes of CD4+, CD8+ cells count and concentration of cytokines
Rate of progression to severe case
Duration of fever
Diagnosis value of C-reactive protein
Absolute value and percentage of peripheral blood lymphocytes
Lung image features improving time
Clinical Symptom Remission time
fever duration
Leicester cough questionaire (LCQ)
minimum oxygen
Negative conversion rate

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Detection of TNF-α levels, IL-10 levels
Detection of immune cells that secret cytokines, including CXCR3+, CD4+, CD8+, NK+ cells, and regulatory T cells (CD4 + CD25 + FOXP3 + Treg cells).
Changes of oxygenation index (PaO2/FiO2) ,blood gas test
Changes of c-reactive protein and calcitonin
SOFA
Serum inflammatory factor concentration
ICU inpatient days
Quality of life assessment in 90 day follow-up
Improving of PSQI
Improving of ISI
Improving of PHQ-9
Improving of GAD-7
Improving of SAS
Lung lesion
Immune function
Time of SARS-CoV-2 clearance
Blood test
SPO2
DLco%
6MWT
HRCT score
Change in cough

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Clinical cure rate

Changes in compound physiological index
Changes in the scores of the St. George's Hospital Respiratory Questionnaire
Changes in modified British Medical Research Council Dyspnea Scale (mMRC) scores
Changes in vital capacity of the lung

Total mortality
The incidence of sudden cardiac death

COVID-19 symptom score
Integral evaluation of qi deficiency and phlegm stasis syndrome
Pulmonary function assessment
St George's respiratory questionnaire score
Occurrence of adverse events
Occurrence of serious adverse reaction
Anti SARS-CoV-2 S IgG antibody response(ELISA)
Neutralizing antibody response to SARS-CoV-2
Neutralizing antibody response to Ad5-vector
Recovery time
Virus nucleic acid negative time and rate
The rate and time of main symptoms (fever, fatigue and cough) disappear
Pulmonary imaging changes
Arterial oxygen partial pressure and oxygenation index were measured 2 days after infusion
Incidence of abnormal indicators of liver and kidney function, blood routine and urine routine
Rate of adverse reaction and adverse event
Incidence of Serious Adverse Events (SAE)
Four-fold growth rate and antibody level (GMT, GMI) of serum antibody against COVID-19
Four-fold growth rate and antibody level (GMT, GMI) of COVID-19 antibody
pulmonary function
mMRC dyspnea scale
strength of lower limber muscles
ICF-COVID simple evaluation form
Quality of life

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Safety indexes of adverse reactions
Safety indexes of Incidence of abnormal laboratory index
Safety indexes of serious adverse events (SAEs)
Immunogenicity indexes of the seropositive rate, GMT, and GMI of neutralizing antibodies
Immunogenicity indexes of the seroconversion rate, seropositive rate, GMT and GMI of

body temperature
symptom

Covid-19 symptoms
TCM Constitution
Total IgG
Total IgA
IFN-α
Activity endurance
IL-6 level
Blood routine biochemical indicators
LUNG CT scan
microparticle
Lung CT imaging findings
Peripheral blood CD4+ cell count
blood lactic acid
oxygen saturation
The virological clearance rate of throat swabs, sputum, or lower respiratory tract secretions at
The proportion of responder at week 8
The change in the average VAS score of main symptoms from baseline
The change in the severity VAS score of main symptoms from baseline
The change in the average VAS score of other symptoms from baseline
The change in mean frequency of diarrhea and spontaneous bowel movements from baseline
Dynamic oxygen saturation
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SARS-CoV-2 nucleic acid test and 2019-nCoV Antibody Test
PHQ-9
GAD-7

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Specific antibody IgM, IgG generation time
Changes in peripheral blood white blood cell counts, biochemical indicators, and blood coagulation from baseline on day 1, day 5, day 10, and day 15 after medication
Changes in C-reactive protein from baseline on day 1, day 5, day 10, and day 15 after medication

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Quality of Life
6MWD
Lung function
Chest CT examination
Reversion rate of viral nucleic acid
Percentage of patients need for non-invasive ventilation or HFNC
Percentage of patients need for invasive ventilation
Negative conversion time of SARS-COV-2 nucleic acid test
Time for relieving fever
Time for improvement of respiratory symptoms
Fatigue scale-14
Patient Health Questionnaire
Clinical Respiratory Symptom Scale
Vital signs

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Incidence of abnormal indicators of liver and kidney function, blood routine and urine routine
Incidence of adverse reactions/events
Incidence of Serious Adverse Events (SAE)
Four-fold growth rate and antibody level (GMT, GMI) of serum antibody against SARS-CoV-2
Four-fold growth rate and antibody level (GMT, GMI) of SARS-CoV-2 antibody

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2019-nCoV Antibody Test
Sars-cov-2 viral load levels in Oral / nasopharyngeal swab, anal swab, sputum, blood
Leukocyte, neutrophil, lymphocyte counts
lung CT
PBMC lymphocyte flow cytometry

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Fasting blood glucose
Liver function
Renal function
Electrolyte
Muscle enzyme
proportion of subjects change from mild or moderate type to severe type
proportion of subjects change from severe type to critical type
on days 7, 14 and 21, novel coronavirus nucleic acid conversion rate
Novel coronavirus nucleic acid negative conversion time
The time and proportion of improvement in pulmonary imaging
Safety index-incidence rate of adverse reactions
Safety index-incidence rate of serious adverse events in phase Ⅰ
Safety index-incidence rate of serious adverse events in phase Ⅱ
Immunogenicity index-seropositive rate, GMT, and GMI of neutralizing antibodies
Immunogenicity index-seroconversion rate, seropositive rate, GMT, and GMI in phase Ⅰ
IL-6 (ng/ml)
IL-10 (ng/ml)
IL-1β (ng/ml)
TNF-α (pg/ml)
leukocyte count (10^9/l)

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Time and rate of coronavirus become negative
Clearance rate and time of main symptoms (fever, fatigue, cough)
Exacerbation (transfer to RICU) time
Routine blood test
Biochemical Indicators
Serious adverse events
Level of Neutralizing antibodies against SARS-CoV-2 Phase Ia (Day 0, 14 schedule)
Level of Neutralizing antibodies against SARS-CoV-2 Phase IIa (Day 0, 14 schedule)
Level of IgG antibodies against SARS-CoV-2 Phase Ia (Day 0, 14 schedule)
Level of IgG antibodies against SARS-CoV-2 Phase IIa (Day 0, 14 schedule)

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Treatment-to-fever time (for patients with fever at admission)
Time between treatment and remission of cough (patients with moderate to severe cough when admitted)
Time from treatment to dyspnea relief (defined as mild or no dyspnea, applicable to patients with moderate dyspnea at admission)

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Primary pharmacokinetic variables
Primary pharmacokinetic variables
Primary pharmacokinetic variables
Primary pharmacokinetic variables
Clinical remission rate
Time and rate of laboratory indicators related to disease improvement to return to normal
Rate of patients with fever
Rate of patients with dyspnea
Rate of patients with requring supplemental oxygen

Immunogenic end point

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Specific IgG antibody
Neutralizing antibody
Cellular immunity
Neutralizing antibody GMT, positive rate
Neutralizing antibody GMT, positive conversion rate/positive rate
Neutralizing antibody GMT
Neutralizing antibody GMT
Neutralizing antibody GMT, positive rate

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Serious adverse events
Seroconversion rate of Neutralizing antibodies against SARS-CoV-2 Phase Ib
Seroconversion rate of IgG antibodies against SARS-CoV-2 Phase Ib
Level of Neutralizing antibodies against SARS-CoV-2 Phase Ib
Level of IgG antibodies against SARS-CoV-2 Phase Ib

Serum Concentration of BRII-196

Serum Concentration of BRII-198

AUC0-t
AUC0-∞
t1/2
Tmax
Anti-drug antibody(ADA)
six-minute walk test
pulmonary function recovery time
clinical symptoms
Lung CT recovery rate and recovery time
Novel coronavirus nucleic acid and antibody changes

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The proportion of subjects experiencing serious adverse events (SAEs), occurring up to Day 21 after prime vaccination and Day 28 after boost vaccination, up to Month 3, 6 and 12.
The proportion of subjects experiencing AE associated with BNT162b1, occurring up to Month 3, 6 and 12.
The proportion of subjects experiencing abnormal markers of hematology, blood chemistry and

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Occurrence of adverse events (AE) within 7 days after each dose of the recombinant SARS-CoV-2 vaccine (Sf9 cell) or placebo.
Occurrence of AE up to Day 28 after prime and boost vaccination.
The proportion of SAEs up to Day 28 after prime and boost vaccination.
The proportion of SAEs up to Month 12 after prime and boost vaccination

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Changes in laboratory indicators before vaccination and 3 days after vaccination (blood routine, blood biochemistry, coagulation function and urine routine)

Humoral immune response

Safety index-incidence of serious adverse events
Immunogenicity index-seropositive rates of neutralizing antibody
Immunogenicity index-geometric mean titer (GMT) of neutralizing antibody
Immunogenicity index-geometric mean ratio (GMR) of neutralizing antibody
TCM syndrome curative effect index
Evaluation criteria of TCM for osteonecrosis of the femoral head
General physical signs
Electrocardiogram
Laboratory physical and chemical indicators
TCM curative effect index
Evaluation criteria of TCM for osteonecrosis of the femoral head
General physical signs
Electrocardiogram
Laboratory physical and chemical indicators
Safety indexes of adverse events
Safety indexes of Hematological examination measures (Hemoglobin, WBC)
Safety indexes of Blood routine measures(ALT, AST)
Safety indexes of SAE
Immunogencity indexes of GMT
PK Measures:AUC0-tau
PK Measures：Cmax
PK Measures：Tmax
PK Measures：t1/2
PK Measures：Cl
Summary of the World Health Organization quality of Life scale
Score of main symptoms and signs
Pulmonary function
Chest CT examination
Serious Adverse Events(SAE)
Abnormal indicators of blood routine, blood biochemistry, urine routine and coagulation function
The seropositive rates and level of SARS-CoV-2 neutralizing antibody and IgG antibody(tested by ELISA)

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(VO2max)
(VO2max)
(VE /VCO2)
(VE /VCO2)
(VE /VCO2)

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Incidence of adverse reactions/events
Serious Adverse Events (SAE)
Level of SARS-CoV-2 IgG antibody (tested by ELISA)
The seropositive rates and level of SARS-CoV-2 neutralizing antibody and IgG antibody (tested by ELISA)
Immunogenicity index-Seroconversion rate of the neutralizing antibody
Immunogenicity index-Seropositive rate of the neutralizing antibody
Safety index-Incidence of adverse reactions
Safety index-Incidence of serious adverse events
Local and systemic adverse reactions/events within 14 days after each dose
All adverse reactions/events within 42 days after each dose
Serious adverse events (SAE) during the entire study period (12 months after the full vaccination)

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SARS-CoV-2 serum neutralizing titers - SCR
SARS-CoV-2 serum neutralizing titers - GMT
SARS-CoV-2 anti-S1 immunoglobulin G (IgG) antibody level - SCR
SARS-CoV-2 anti-S1 IgG antibody level - GMT
SARS-CoV-2 serum neutralizing antibody level - Geometric mean fold rise (GMFR)
The proportion of adverse events (AE) within 7 days after each dose of the recombinant SARS-CoV-2 vaccine (CHO cell) or placebo.
The proportion of abnormal markers of hematology, blood chemistry and urine analysis within 3 days after each dose of the recombinant SARS-CoV-2 vaccine (CHO cell) or placebo
The proportion of serious adverse events up to Month 12 after prime and boost vaccination
The incidence of adverse events (AE)
The incidence of severe adverse events (SAE)
The incidence of serious adverse events
Geometric mean (GMT) of anti-SARS-CoV-2-specific neutralizing antibodies
The positive conversion rate of S-RBD protein-specific antibody

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Time of disappearance of fever symptoms
The rate of conversion from COVID-19 positive to COVID-19 negative
Time of disappearance of cough
Respiratory rate
Blood oxygen saturation
routine blood test
blood biochemical
routine urine test
Blood oxygen
Proportion of subjects with infusion-related reactions
Proportion of subjects with hypersensitivity reactions
Mean of change from pre-dose baseline in ECG readings
Mean of change from pre-dose baseline in WBC count.
Mean of change from pre-dose baseline in RBC count
The specific immune cell level of SARS-CoV-2 in whole blood 28 days after the last dose of vaccination
Local and systemic adverse reactions/events within 7 days and 6 months after each dose
Serious adverse events (SAE) during the entire study period (6 months after the full vaccination
Serum neutralizing antibodies, IgG antibodies, and IgM antibodies of SARS-CoV-2 after enhancing immunity of vaccination
The specific immune cell level of SARS-CoV-2 in whole blood 28 days after the last dose of vaccination
Local and systemic adverse reactions/events within 7 days and 6 months after each dose
The specific immune cell level of SARS-CoV-2 in whole blood 28 days after the last dose of vaccination
Serum neutralizing antibodies, IgG antibodies, and IgM antibodies of SARS-CoV-2 after enhancing immunity of vaccination
Local and systemic adverse reactions/events within 7 days and 6 months after each dose

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Improvement time of clinical symptoms(disappearance of pulmonary rales, recovery of body temperature, absorption of pulmonary lesions> 50%, recovery of blood picture)
Changes of oxygenation index in blood gas analysis (up to 28 days)
To turn a standard time in the ICU(up to 28 days)
Duration (days) of mechanical ventilation (up to 28 days)

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Single symptom disappearance rate and main symptom disappearance time
Changes in color, quality and quantity of sputum in patients with expectoration
Changes in oxygenation index (OI)
Proportion of aggravation during treatment (in line with the definition of severe or critical illness in the treatment scheme)
Clinical symptoms and severity during the isolation observation period
Clinical symptoms appearance time and proportion during the isolation observation period
Proportion of mild and common cases of novel coronavirus pneumonia diagnosed during the isolation observation period

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The incidence of adverse events (AE)
The incidence of grade 3 adverse events (AE)
The incidence of severe adverse events (SAE)
Geometric mean (GMT) of specific antibody
Geometric mean (GMT) of anti-SARS-CoV-2-specific neutralizing antibodies
TCM syndrom score
PHQ-9 score
GAD-7 score
routine blood test
Time of occurrence of positive nucleic acid tests in nasal and throat swabs during the isolation medical observation period
Proportion of asymptomatic infections among positive nucleic acid tests
Proportion of mild, normal, and severe patients with positive nucleic acid tests
Date of onset during the isolation medical observation period (that is, the time when the clinical
Hamilton Anxiety Scale
Patient Health Questionnaire-15
Scale of severity of insomnia
Adverse Reaction Scale
Duration of COVID-19 related symptoms through Day 29 among subjects with mild-moderate COVID-19
Proportion of subjects who have mild-moderate COVID-19 and develop severe COVID-19 after randomization
Assessment of PK parameters (Group A and B): maximum serum concentration observed

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Safety index-incidence of adverse reactions within 56 days after the first dose vaccination
Safety index-incidence of serious adverse events
Immunogenicity index-seropositive rates of neutralizing antibody against SARS-CoV-2
Immunogenicity index-geometric mean titer (GMT) of neutralizing antibody against SARS-CoV-2

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The incidence of adverse events (AE)
The incidence of severe adverse events (SAE)
The Geometric mean titer (GMT) of anti-SARS-CoV-2 specific neutralizing antibody (euvirus and pseudovirus neutralization assays)
The Geometric mean fold increase (GMI) of anti-SARS-CoV-2 specific neutralizing antibody
Routine blood
Biomedical indexes
Blood biochemical
The length of time
Imaging CT changes
Incidence of adverse reactions within 28 days after vaccination.
Incidence of adverse events within 28 days after vaccination.
Incidence of unsolicited AE within 28 days after vaccination.
Incidence of serious adverse events(SAE) from the first dose to the 6 months after completing the last dose of vaccination

Safety outcomes
Immunogenicity outcomes

Immunogenicity

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Incidence of Adverse Events/Reactions (AE/AR)
Incidence of Adverse Events/Reactions (AE/AR)
Changes in laboratory test indicators
GMT of Covid-19 S protein RBD specific antibody
Seroconversion rate of Covid-19 S protein RBD specific antibody
Geometric Mean Titer (GMT) of SARS-CoV-2 Specific Neutralizing Antibody (Live SARS-CoV-2 neutralization assay) On the 30th day after the full course of vaccination, the neutralizing antibody level (wild Strains)
Seroconversion and Geometric Mean Titer (GMT) of SARS-CoV-2-Specific Binding Antibody (IgG) Before each dose, the 15th day after the second dose, the 15th, 30th, 90th, 180th, 360th
The Geometric Mean Titer (GMT) of anti-SARS-CoV-2 neutralizing antibody
The four-fold increase rate of anti-SARS-CoV-2 neutralizing antibody
The Geometric Mean Titer (GMT) of anti-SARS-CoV-2 neutralizing antibody
The four-fold increase rate of anti-SARS-CoV-2 neutralizing antibody
Immune Persistence

immunogenicity index
Immunogenicity indicators

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Safety index-incidence of adverse reactions
Safety index-incidence of serious adverse events

Adverse reaction

Adverse events following vaccination

TCM syndrome score
Depression Scale (PHQ-9), anxiety scale (GAD-7)
Blood oxygen saturation
6 minutes walking distance
Chest CT

the death in hospital
the composite endpoint of aggravation

Seroconversion rate of SARS-CoV-2 neutralizing antibody
GMI of SARS-CoV-2 neutralizing antibody
GMT of SARS-CoV-2 S protein RBD antibody
Seroconversion rate of SARS-CoV-2 S protein RBD antibody
GMI of SARS-CoV-2 S protein RBD antibody
Incidence of solicited AE within 14 days after the booster dose
Incidence of unsolicited AE within 28 days after the booster dose.
Incidence of serious adverse events (SAE) till the 6 months after the booster dose.
GMT of binding antibodies against SARS-CoV-2 S and N protein on day 14, day 28 and month 6 after the booster dose
Local and systemic adverse reactions / events within 14 days after each dose
All adverse reactions / events within 42 days after each dose
Severe adverse events (SAE) during the whole study period (12 m after the whole course of vaccination)
Virus specific antibody
Virus neutralizing antibody
Indicators of blood infectious diseases
Blood routine test
Blood biochemical index
Nucleic acid negative time
The ratio of nucleic acid to negative
Major symptoms disappeared
CT imaging changes of chest
Hospital stays

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Assess Pharmacokinetic parameters of LY-CovMab
Assess Pharmacokinetic parameters of LY-CovMab
Assess Pharmacokinetic parameters of LY-CovMab
Assess Pharmacokinetic parameters of LY-CovMab
Assess Pharmacokinetic parameters of LY-CovMab
Seroconversion rate of the neutralizing antibody to SARS-CoV-2
Seropositivity rate of the neutralizing antibody to SARS-CoV-2
GMT of the neutralizing antibody to SARS-CoV-2
GMI of the neutralizing antibody to SARS-CoV-2
Geometric mean concentrations of anti-pneumococcal antibody for 23 serotypes (ELISA)

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Incidence of serious AEs (SAEs) and adverse events of special interest (AESIs)
Proportion of subjects with abnormal markers of hematology, biochemistry, urinalysis and coagulation parameters
Geometric mean titer (GMT) of SARS-CoV-2 neutralising antibodies
Seroconversion rate (SCR) of SARS-CoV-2 neutralising antibodies
Percentage of participants reporting adverse events (AEs)
Percentage of participants reporting solicited Aes
Percentage of participants reporting unsolicited Aes
Percentage of participants reporting serious adverse events (SAEs)
Percentage of participants reporting adverse events of special interest (AESIs)
Immunogenicity index of seroconversion rate of the neutralizing antibody
Immunogenicity index of seropositivity rate of the neutralizing antibody
Immunogenicity index of GMI of the neutralizing antibody
Immunogenicity indexes of seroconversion rate of HI antibody for each of the 4 virus strains
Immunogenicity indexes of seroprotection rate of HI antibody for each of the 4 virus strains
Incidence of adverse reactions/events
Seroconversion rate of SARS-CoV-2 IgG binding antibody
Geometric mean titer of SARS-CoV-2 IgG binding antibody
Geometric mean titer of SARS-CoV-2 IgG binding antibody
Geometric mean titer of SARS-CoV-2 neutralizing antibody
Incidence of Serious Adverse Events (SAE)
Incidence of abnormal indicators of laboratory safety examinations(including blood routine, blood biochemistry, urine routine, thyroid function and coagulation function)
Geometric mean titer of SARS-CoV-2 neutralizing antibody
Geometric mean titer of SARS-CoV-2 IgG binding antibody
Seroconversion rate of the neutralizing antibody to live SARS-CoV-2 in total population
Seropositivity rate of the neutralizing antibody to live SARS-CoV-2 in total population
GMT of the neutralizing antibody to live SARS-CoV-2 in total population
Geometric mean increase (GMI) of the neutralizing antibody to live SARS-CoV-2 in total population

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Phase I/II clinical trial：The incidence of adverse events (AEs)
Phase I/II clinical trial：The incidence of adverse events (AEs) in all participants.
Phase I/II clinical trial：The incidence of serious adverse events(SAEs) in all participants.
Phase I/II clinical trial：The incidence of serious adverse events(SAEs) in all participants.
Phase I clinical trial：Changes in laboratory test indicators

Humoral immunity (IgG, neutralizing antibody)
cellular immunity

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Nucleic acid negative time
7-day/14-day nucleic acid conversion ratio
The ratio of main symptoms disappeared in 7 days/14 days
7 days/14 days chest CT imaging changes
Hospital stays

Symptoms
Body temperature
Patients status after the COVID-19 confirmed diagnosis

Netralizing antibodies against the adenovirus vector(ChAdTS)
Cellular-mediated immunity

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Occurrence of AE ≥ Grade 3
Occurrence of AEassociated with vaccination >=Grade 3
The incidence of SAE

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6 minute walk distance
strength of lower limber muscles
Quality of life
mMRC dyspnea scale
the power of gripping
Virus negative conversion rate
Antipyretic rate
Symptom relief time
Finger oxygen improvement rate
Disease progression rate
Changes in the patients' clinical status through the WHO Clinical Progression Scale (scores 0-10) at day 7
Changes in the patients' clinical status through the WHO Clinical Progression Scale (scores 0-10) at day 10
Ventilator usage rate, usage time at day 7
6min walk
PHQ-9
Respiratory symptoms
immune cells
Liver and renal function test
Incidence of adverse events within 0-28 days after the booster dose.
Incidence of serious adverse events (SAE) till the 12 months after the booster dose.
GMT of neutralizing antibodies against live SARS-CoV-2 virus on day 7 and 28 after the booster dose.
Fold increase and seroconversion of neutralizing antibodies against live SARS-CoV-2 virus on
6 minute walk distance
strength of lower limber muscles
Quality of life
mMRC dyspnea scale
Incidence of solicited AE within 14 days after the booster dose
Incidence of unsolicited AE within 28 days after the booster dose.
Incidence of serious adverse events (SAE) till the 6 months after the booster dose.
GMT of binding antibodies against SARS-CoV-2 S and N protein on day 14, day 28 and month 6 after the booster dose

Safety index-incidence of serious adverse events
Seroconversion rate,seropositivity rate and GMI of the neutralizing antibody to SARS-CoV-2
Seropositivity rate of IgG antibody

Adverse events following vaccination

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All cause mortality ascertained from data analysed to day 28
Ventilator-free days within 28 days
Negative conversion rate of SARS-CoV-2 nucleic acid in on days 14 after randomization
Average length of hospital stay
Immunogenicity index-seropositive rate of neutralizing antibodies
Immunogenicity index-seroconversion rate of neutralizing antibodies
Immunogenicity index-GMI of neutralizing antibodies (CZ02 strain)
Immunogenicity index-seropositive rate of neutralizing antibodies
Immunogenicity index-seroconversion rate of neutralizing antibodies

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Proportion of patients who progress to severe disease
Duration of fever
C reactive protein concentration in peripheral blood
Absolute value and percentage of peripheral blood lymphocytes
Time of improvement of chest imaging

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Antibody IgM, IgG levels
The neutralizing antibody S, N, total antibody
Memory B cell ratio
Imaging examination
electrocardiogram

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CD44 factor
VII factor activity
uPA / uPAR dynamic change
SARS-cov-2 neutralizing antibody GMT on day 14 after full immunization with recombinant Novel Coronavirus vaccine (CHO cells);
HI antibody GMT, positive conversion rate and serum protection rate of influenza virus typing 30 days after influenza vaccination;
Incidence of adverse reactions/events at each time point after each dose of recombinant Novel

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Neutralizing antibody as measured by pseudovirus neutralization assay
IgG antibody

Immunogenicity index-Seropositivity rate of the neutralizing antibody to live SARS-CoV-2
Immunogenicity index- GMT of the neutralizing antibody to live SARS-CoV-2
Immunogenicity index-GMI of the neutralizing antibody to live SARS-CoV-2
Immunogenicity index-Seropositive rate of the neutralizing antibody to EV71
Immunogenicity index-GMT of the neutralizing antibody to EV71

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Geometric mean increase (GMI) of the neutralizing antibody to live SARS-CoV-2
Seroconversion rate of the neutralizing antibody to live SARS-CoV-2
Seropositivity rate of the neutralizing antibody to live SARS-CoV-2
Incidence of adverse reactions 0-7 days after vaccination
Incidence of adverse reactions 0-28 days after vaccination
Incidence of solicited adverse events within 7 days after each dose.
Incidence of adverse events within 28 days after each dose.
Incidence of unsolicited adverse events within 28 days after each dose.
Incidence of serious adverse events (SAE) till the 3 months after the last vaccination.
GMT of anti-S protein of SARS-CoV-2 binding antibodies on day 28 and month 3 after the last

Safety index-Incidence of adverse reactions
Safety index-Incidence of adverse reactions
Safety index-Incidence of SAEs and AESIs

Immunogenicity index-Seropositivity rate of neutralizing antibodies(CZ02 strain)

maximum concentration (Cmax)
area under the concentration-time curve (AUC)
half-life (t1/2)
clearance (CL)
volume of distribution (V)

Netralizing antibodies against the adenovirus vector(ChAdTS)
Cellular-mediated immunity
Neutralizing Response against Variants

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Pharmacokinetics: AUC
Pharmacokinetics: Tmax
Pharmacokinetics: Cmax
Pharmacokinetics: Vd
Pharmacokinetics: CLt
IgG antibody concentration
T-cell response level
Serum neutralizing antibody positive rate
Serum neutralizing antibody positive conversion rate
Intestinal flora
IgG antibody concentration
T-cell response level
Serum neutralizing antibody positive rate
Serum neutralizing antibody positive conversion rate
Intestinal flora

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Illness Perception

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Incidence of adverse reactions within 30 minutes after the booster dose.
Incidence of adverse reactions within 14 days after the booster dose.
Incidence of adverse events within 28 days after the booster dose.
Incidence of serious adverse events (SAE) till the 6 months after the booster dose.
Fold increase and seroconversion of neutralizing antibodies against live SARS-CoV-2 virus on

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Pharmacokinetic parameters(Cmax)
Pharmacokinetic parameters(AUC0-τ)

The seropositive rate of neutralizing antibodies
GMT of neutralizing antibodies
GMFR of neutralizing antibodies
severe adverse events and adverse events of special interest
Phase I: GMT and seroconversion rate of total IgG antibody (ELISA method) against the Alpha, Beta, Delta variants SARS-CoV-2 and T4-Foldon on Day 28 after the first dose of vaccination;
Phase I: GMT and seroconversion rate of total IgG antibody (ELISA method) against the Alpha, Beta, Delta variants of SARS-CoV-2 and T4-Foldon on Day 14, Day 28, Day 90, Day

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Blood gas analysis
mMRC
Borg

Motor function

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Measure time point of outcome and Measure method of Current Secondary outcome measures (If Specified)

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The difference of PaO2/FiO2 between two groups [Time Frame: 7 days after randomization]
PaO2/FiO2 denotes ratio of arterial partial pressure of O2 and the fraction of inspired oxygen, with a higher PaO2/FiO2 means favorable outcome.
Lower Sequential Organ Failure Assessment (SOFA) score [Time Frame: 7 days
The time to fever resolution rate [Time Frame: 14 days]
Time to complete remission of fever in eligible subjects
Time to recovery of lung injury [Time Frame: 14 days]
improvement of chest radiographic evidence indirectly reflects recovery in patients infected with COVID-19

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The virological clearance rate of throat swabs, sputum, or lower respiratory tract secretions at day 3 [Time Frame: 3 days after randomization]
The virological clearance rate of throat swabs, sputum, or lower respiratory tract secretions at day 5 [Time Frame: 5 days after randomization]
Number of participants with treatment-related adverse events as assessed by CTCAE

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The disease prorogation-temperature [Time Frame: Day 0 till day 21]
Body temperature will be followed everyday during time frame.
The disease prorogation-respiratory function 1 [Time Frame: Day 0 till day 21]
Respiratory rate will be followed everyday during time frame.
The disease prorogation-respiratory function 2 [Time Frame: Day 0 till day 21]

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All cause mortality [Time Frame: up to 28 days]

baseline SpO2 during screening, PaO2/FiO2 <300mmHg or a respiratory rate ≥ 24 breaths per min without supplemental oxygen

Frequency of respiratory progression [Time Frame: up to 28 days]

Defined as SPO2≤ 94% on room air or PaO2/FiO2 <300mmHg and requirement for

Clinical status [Time Frame: days 7, 14, 21, and 28]

Clinical status, assessed by the ordinal scale at fixed time points (days 7, 14, 21, and 28).

Time to Hospital Discharge OR NEWS2 (National Early Warning Score 2) of ≤ 2 maintained for 24 hours [Time Frame: up to 28 days]

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Rate of no fever [Time Frame: two weeks]
Rate of respiratory symptom remission [Time Frame: two weeks]
Rate of lung imaging recovery [Time Frame: two weeks]
Rate of CRP,ES,Biochemical criterion (CK,ALT,Mb)recovery [Time Frame: two weeks]
Rate of no fever [Time Frame: two weeks]
Rate of respiratory symptom remission [Time Frame: two weeks]
Rate of lung imaging recovery [Time Frame: two weeks]
Rate of CRP,ES,Biochemical criterion(CK,ALT,Mb) recovery [Time Frame: two weeks]
Rate of normal tempreture [Time Frame: day 7]
Rate of patients without fever at day 7
Rate of respiratory symptom remission [Time Frame: day 7]
Rate of patients with respiratory symptom remission at day 7
Rate of lung imaging recovery [Time Frame: day 7]

time till the SARS-CoV-2 clearance [on day 3, day 6, day 9, et al; RT-PCR]

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Time of clinical remission [Time Frame: 28 days]
The definition of clinical remission:
Based on the symptoms of the disease (fever,cough,diarrhea,myalgia,dyspnea) has been relieved for 48 hours;
There is no evidence of disease progression(New dyspnea, SpO2
28-days mortality [Time Frame: on the day 28 after enrollment]
wether the patient survives
ICU length of stay [Time Frame: on the day 28 after enrollment]
days of the patients staying in the ICU
Demand for first aid measuments [Time Frame: on the day 28 after enrollment]

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Number of participants with treatment-related adverse events as assessed by CTCAE v5.0 [Time Frame: 14 days after randomization]
The critical illness rate of subjects at weeks 2 [Time Frame: 14 days after randomization]
The diagnosis of critical illness case was based on the notice on printing and

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blood pressure
heart rate [baseline, 1-week, 2-week, 3-week, 4-week;]
immunoglobin level

Others (liver and kidney function, muscle enzymes, myoglobin)[Day 1, 3, 5, 7, 10, 14, 21, 28;]

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rate of composite advers outcomes.[; Defined as SPO2≤ 93% without oxygen supplementation, PaO2/FiO2 <=300mmHg or a respiratory rate ≤30 breaths per min without supplemental oxygen (Time Frame: 14 days)]
rate of no fever (Time Frame: 14 days)
rate of no cough

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COVID-19 RNA negative conversion rate[at the end of the treatment, on the 28th day of treatment;]
Disappearance rate of main symptoms(fatigue, dyspnea)[at the end of the treatment, on the 28th day of treatment;]

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Time of virus nucleic acid test negative [Time Frame: Measured from day 0 through day 28]
Marker for 2019-nCoV
CD4+ and CD8+ T cell count [Time Frame: Measured from day 0 through day 28]
Marker of immunological function

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Arterial Oxygen Saturation[Measured once at the time of enrollment and at 6 o'clock and 14 o'clock each day after administration.;]
Critical (severe or critical) conversion rate
Inflammation index (CRP)[Once at the time of enrollment and 3, 7, and 14 days after administration (4 times in total).;]
time to temperature normal[time to temperature normal; temperature measure]
breath symptoms and measure[during therapy; CT]
hospitalization days [to out hospital; count]

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Side effects in the UC-MSCs treatment group [Time Frame: From Baseline (0W) to 96 week after treatment]
Incidence of acute and chronic treatment-related adverse events in patients with novel coronavirus severe pneumonia receiving UC-MSCs infusion as assessed.
28-days survival [Time Frame: Day 28]

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Clinical remission rate [Time Frame: within 14 days from the start of medication]
Clinical remission was defined as (one of them): sustained (more than 48 hours) alleviation of illness based on symptom (fever, cough, dyspnea, myalgia, diarrhea and so on) all being absent and no evidence for progression.
Dynamic changes of oxygenation index [Time Frame: within 14 days from the start of

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clinical cure incidence in 14 days [Time Frame: 14 days]
The clinical symptoms and signs improved or alleviated (the temperature be normal , respiratory symptoms improved significantly, imaging showed obvious absorption) and no additional or alternative treatment was needed.
the duration of virus change to negative [Time Frame: 30 days]

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The improvement proportion of pulmonary inflammation [Time Frame: Week 24]
Evaluation of pulmonary inflammation Improvement
The improvement proportion of clinical symptom [Time Frame: Week 24]
Discomfort symptoms include dyspnea, cough, exhausted, fatigue, insomnia, sweating, poor appetite, diarrhea, etc., which are common manifestations of patients

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fever[evryday; Temperature measurement]
feeble[evryday; Subject self-report]
novel coronavirus nucleic acid detection
Lung CT
Day 7, 14, 21, 28 clinical status[Day 7, 14, 21, 28; Evaluated by Grade 6 rating scale]
Time from study entry to discharge
Frequency of serious adverse drug reactions
Dynamic changes of laboratory indexes[Baseline, 7 days, 14 days, 21 days, 28 days; Blood]
Time to COVID-19 RNA convert negative
Other symptoms relief rate & time[; Weak/hacking cough]
Time & Rate of inflammatory biomarkers convert normal
Time to recovery
Frequency of antipyretic use

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Time to CoVID-19 RT-PCR negative in upper respiratory tract specimen [Time Frame: up to 28 days]
Change (reduction) in CoVID-19 viral load in upper respiratory tract specimen as assessed by area under viral load curve. [Time Frame: up to 28 days]
Time to defervescence (in those with fever at enrolment) [Time Frame: up to 28 days
The incidence of common Novel coronavirus pneumonia progressing to severe[Un the 5th, 10th day after the first dose, and discharge or turn to severe; Patients with Novel coronavirus pneumonia who meet any of the following are severe patients: respiratory distress, RR ≥30 beats / min; in a resting state, means oxygen saturation <93%; arterial blood oxygen pressure (PaO2) / oxygen concentration (FiO2))

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P/F ratio over time [Time Frame: 14 days]
PaO2/FiO2 ratio
Sequential organ failure assessment score(SOFA score) over time [Time Frame: 14 days]
SOFA, including assessment of respiratory, blood, liver, circulatory, nerve, kidney
Time to 2019-nCoV RT-PCR negativity in upper and lower respiratory tract specimen[Baseline, 7 days, 14 days, 21 days, 28 days;]
clinical status[Day 7, 14, 21, 28;]
Time from study entry to discharge
Frequency of serious adverse drug reactions

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Body temperature [Every week; Auxiliary temperature]
3CL Mpro of Coronavirus[Every week; Real - time PCR method]
Routine blood test[Every week;]
Routine urine test[Every week;]
Liver function test[Every week;]

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Inflammatory markers[Within 14 days;]
Clinical symptoms[Within 14 days;]
Liver and kidney function[Within 14 days;]

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Change in lesion proportion (%) of full lung volume from baseline to day 10 and 90 [Time Frame: Day 10, Day 90]
Evaluation of Pneumonia Improvement
Change in consolidation lesion proportion (%) of full lung volume from baseline to day 10, 28 and 90. [Time Frame: Day 10, Day 28, Day 90]
Rate of mortality within 28-days [Time Frame: At baseline, Day 1, Week 1, Week 2, Week 4, Week 8]
Marker for efficacy of treatment
Sequential organ failure assessment [Time Frame: At baseline, Day 1, Week 1, Week 2, Week 4, Week 8]

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Within 4 weeks of death from respiratory causes
Within 4 weeks of disease progression or death
Routine lymphocyte recovery[4 weeks;]
Changes in inflammatory markers of blood (il-8 et al.)[4 weeks;]
Absolute change in viral nucleic acid from baseline[4 weeks;]

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The incidence rate of complication[Long-term follow-up was 1 year; Visiting]

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The fading and obviously improving time of clinical symptoms (fever, dry cough, fatigue, etc)
The negative conversion rate of SARS-CoV-2 nucleic acid[1, 3, 7 and 14 days after infusion;]
Negative conversion time of SARS-CoV-2 nucleic acid
CRP[day1, day7 and day14;]
IL-6[day1, day7 and day14;]
TNF-α [Before and after 14 days of treatment;]
Time of improvement of clinical symptoms (fever, cough)
Adverse drug reactions[within 14 day;]

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Treatment compliance[Enrollment and two weeks after enrollment; The patient's degree of cooperation with the physician's treatment plan]
To evaluate the difference in isolation and rehabilitation of patients after discharge[7 days and 14 days after discharge; Patients were followed up for isolation and physical recovery]

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body temperature [Time Frame: 2 weeks]
inflammatory indicator

Level of IL2, IL4, IL6, IL10, TNF-alpha and IFN-gama[Everyday; ELISA]
Routine blood test[Everyday; Five-Part Differential Complete Blood Count]
lymphocyte subsets[Everyday; Flow Cytometer]

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All-cause death[28 days;]
Adverse events

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Viral nucleic acid test negative conversion rate and days from positive to negative [Time Frame: 14 days after taking medicine]
Duration of fever [Time Frame: 14 days after taking medicine]
Lung imaging improvement time [Time Frame: 14 days after taking medicine]
Mortality rate because of Corona Virus Disease 2019 [Time Frame: 3 months]

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Lung function
Arterial blood gas analysis[Days 3, 7 and 14;]

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PaO2 [Time Frame: up to 30 days]
blood oxygen saturation (SpO2) [Time Frame: up to 30 days]
clinical status rating on the 7-point ordinal scale [Time Frame: up to 30 days]
The ordinal scale is an assessment of the clinical status at the first assessment of a given study day. The scale is as follows: 1) Death; 2) Hospitalized, on invasive

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Causal mortality[4 week/8week;]
COVID-19 nucleic acid negative conversion rate[4 week/8week;]
All-cause mortality[4 week/8week;]

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Duration of fever [Time Frame: 10 days during the intervention period]
Duration of fever after recruitment
Frequencies of progression to severe illness [Time Frame: 10 days during the intervention period]
Disease is defined as severe if it meets any of the following criteria: 1 Respiratory rate

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Lung function[Base line, week 1, 2, 4 after enrollmen;]
Oxygenation index[Base line, week 1, 2, 4 after enrollment;]

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Clinical outcome [Time Frame: At baseline, 6 hour, Day 1, Day 3,Week 1, Week 2, Week 4, Month 6.]
Improvement of clinical symptoms including duration of fever, respiratory destress, pneumonia, cough, sneezing, diarrhea.
Hospital stay [Time Frame: Baseline through 6 months]

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Clinical Symptom Remission time [Time Frame: The day from admission to clinical remission,up to 2 weeks.]
Clinical Symptom Remission was defined as: The patient's body temperature was normal for more than 72 hours (without taking antipyretics or hormones) and had no conscious dyspnea or decreased dyspnea

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Detection of TNF-α levels, IL-10 levels [Time Frame: 1,3,6,12months]
Cytokines level
Detection of immune cells that secret cytokines, including CXCR3+, CD4+, CD8+, NK+ cells, and regulatory T cells (CD4 + CD25 + FOXP3 + Treg cells). [Time Frame: 1,3,6,12months]

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Lung lesion [Time Frame: 1-28 days]
Lung Lesion by CT
Immune function [Time Frame: 1-28 days]
Th1 cytokines: IL-1β, IL- 2, TNF-a, ITN-γ;
Th2 cytokines: IL - 4, IL - 6, IL - 10;

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Clinical cure rate [Time Frame: 5 months]
Definition of clinical cure: The viral load of the respiratory specimen was negative for two consecutive times (the interval between the two tests was greater than or equal to one day), the lung image improved, and the body temperature returned to normal for more than 3 days, and the clinical manifestation improved.
Changes in compound physiological index [Time Frame: 3 months]
Changes in the scores of the St. George's Hospital Respiratory Questionnaire [Time Frame: 3 months]
St. George's Hospital Respiratory Questionnaire range from 0 to 100. 0 stands for no impact on life and 100 stands for extreme impact on life

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Occurrence of adverse events [Time Frame: 0-28 days post vaccination]
Occurrence of serious adverse reaction [Time Frame: 0-6 months post vaccination]
Anti SARS-CoV-2 S IgG antibody response(ELISA) [Time Frame: 0, 14 days and 6 months post vaccination]
Neutralizing antibody response to SARS-CoV-2 [Time Frame: 0 and 6 months post
Recovery time
Virus nucleic acid negative time and rate
The rate and time of main symptoms (fever, fatigue and cough) disappear
Pulmonary imaging changes
Arterial oxygen partial pressure and oxygenation index were measured 2 days after
Incidence of abnormal indicators of liver and kidney function, blood routine and urine routine[The 4th day after each dose of vaccination in phase I;]
Rate of adverse reaction and adverse event[8-28/30 days, 0-28/30 days after inoculation of each dose of vaccine;]
Incidence of Serious Adverse Events (SAE)[Within 12 months from first vaccination to
pulmonary function
mMRC dyspnea scale
strength of lower limber muscles
ICF-COVID simple evaluation form
Quality of life[: SF-12]

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Safety indexes of adverse reactions [Time Frame: 7 days after each dose of vaccination]
Incidence of adverse reactions 7 days after each dose of vaccination
Safety indexes of Incidence of abnormal laboratory index [Time Frame: On the 3th day after each dose of vaccination in phase I]

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Covid-19 symptoms[; Visual Analogue Scale]
TCM Constitution[; TCM Constitution Judgment Scale]
Total IgG
Total IgA
IFN-α

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The proportion of responder at week 8 [Time Frame: baseline, week 8]
The responder is defined as the participants with ≥ 50% average VAS decrease of chest tightness and fatigue compared with baseline. The VAS ranges from 0 to 100.
The change in the average VAS score of main symptoms from baseline [Time Frame: baseline, week 4, week 8]

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Quality of Life[; WHOQOL-BREF]
6MWD
Lung function
Chest CT examination
Reversion rate of viral nucleic acid
Percentage of patients need for non-invasive ventilation or HFNC
Percentage of patients need for invasive ventilation
Negative conversion time of SARS-COV-2 nucleic acid test
Time for relieving fever
Time for improvement of respiratory symptoms [; Cough was quantified by Cough

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Incidence of abnormal indicators of liver and kidney function blood routine and urine routine [The 4th day after each dose of vaccination in phase I;]
Incidence of adverse reactions/events[8-28/30 days, 0-28/30 days after inoculation of each dose of vaccine;]
Incidence of Serious Adverse Events (SAE)[Within 12 months from first vaccination to

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Safety index-incidence rate of adverse reactions [Time Frame: within 7 days after each dose vaccination]
Incidence rate of adverse reactions within 7 days after each dose vaccination
Safety index-incidence rate of serious adverse events in phase I [Time Frame: From the beginning of the vaccination to 6 months after the booster immunization]
IL-6 (ng/ml) [Time Frame: baseline (at admission), day 3,7 and14 after admission or until the date of discharge or date of death from any cause]
evaluate inflammatory response, blood sample collected at 6:00am
IL-10 (ng/ml) [Time Frame: baseline (at admission), day 3,7 and14 after admission or until the date of discharge or date of death from any cause]

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Serious adverse events [Time Frame: 12 months after the second vaccination]
Occurence of Serious adverse events after vaccination
Level of Neutralizing antibodies against SARS-CoV-2 Phase Ia (Day 0, 14 schedule) [Time Frame: 7, 14 and 28 days after the second vaccination]
Level of neutralizing antibodies against SARS-CoV-2 in serum for the Phase Ia

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Primary pharmacokinetic variables [Time Frame: 12 Weeks]
Area under the curve from the time of dosing to the last measurable concentration time t (AUC0-last)
Primary pharmacokinetic variables [Time Frame: 12 Weeks]
Maximum concentration (Cmax)

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Immunogenic end point [Time Frame: Within 6 months after the last dose of vaccination]
All subjects The levels of IL-2, IL-4, IL-5, IL-6 and IFN-γ levels before each vaccination, day 14 after the first vaccination, day 7 after the second vaccination, and 1 and 6 months after the entire vaccination. All subjects before each dose of

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Neutralizing antibody GMT, positive rate [Time Frame: 14 days after inoculation]
GMT and positive rate of neutralizing antibody 14 days after the first dose of pre-vaccination people
Neutralizing antibody GMT, positive conversion rate/positive rate [Time Frame: 14 days after inoculation]

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Serious adverse events [Time Frame: 12 months after the second vaccination]
Occurence of Serious adverse events after vaccination
Seroconversion rate of Neutralizing antibodies against SARS-CoV-2 Phase Ib [Time Frame: 28 days after the second vaccination]
Seroconversion rate of neutralizing antibodies against SARS-CoV-2 in serum for

Serum Concentration of BRII-196 [Time Frame: up to 24 weeks]

Serum Concentration of BRII-198 [Time Frame: up to 24 weeks]

AUC0-t [Time Frame: 12 weeks]
Area under the curve from the time of dosing to the last measurable concentration time t (AUC0-t)
AUC0-∞ [Time Frame: 12 weeks]
Area Under the Concentration Time Curve (AUC) From Time Zero to Infinity (AUC 0-

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The proportion of subjects experiencing serious adverse events (SAEs), occurring up to Day 21 after prime vaccination and Day 28 after boost vaccination, up to Month 3, 6 and 12. [Time Frame: Day 21 after prime vaccination, Day 28 after boost vaccination, and Month 3, 6 and 12]
The proportion of subjects experiencing AE associated with BNT162b1, occurring up

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Occurrence of adverse events (AE) within 7 days after each dose of the recombinant SARS-CoV-2 vaccine (Sf9 cell) or placebo. [Time Frame: 7 days after each dose.]
Occurrence of solicited AE in the subjects within 7 days after each dose of recombinant SARS-CoV-2 vaccine (Sf9 cell) or placebo.
Occurrence of AE up to Day 28 after prime and boost vaccination. [Time Frame: Day

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Humoral immune response[; antibody titer]

Safety index-incidence of serious adverse events [Time Frame: From the beginning of the vaccination to 12 months after the second dose vaccination]
SAE will be collected throughout the clinical trial.
Immunogenicity index-seropositive rates of neutralizing antibody [Time Frame: The 28th day after each dose vaccination and the 12 month after the second dose

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Safety indexes of adverse events [Time Frame: Day 0-7,0-14,0-28 post each vaccination]
Occurrence of adverse reactions post-vaccination
Safety indexes of Hematological examination measures (Hemoglobin, WBC) [Time Frame: pre-vaccination, day 4 post each vaccination]
PK Measures:AUC0-tau [Time Frame: Day 0 to Day 85]
Area under the plasma drug concentration-time curve from time 0 to one dosing interval (AUC0-tau)
PK Measures：Cmax [Time Frame: Day 0 to Day 85]
Maximum plasma drug concentration after administration (Cmax)

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Serious Adverse Events(SAE)[Within 12 months from first vaccination to completion of full vaccination schedul;]
Abnormal indicators of blood routine, blood biochemistry, urine routine and coagulation function[Day 3 after each dose of vaccination;]
The seropositive rates and level of SARS-CoV-2 neutralizing antibody and IgG

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(VO2max) [Time Frame: The change from baseline in maximum oxygen consumption (VO2max) at maximum exercise load at Week 4 of treatment.]
The change from baseline in maximum oxygen consumption (VO2max) at maximum exercise load at Week 4 of treatment.
(VO2max) [Time Frame: The change from baseline in maximum oxygen consumption

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Incidence of adverse reactions/events[0-28 days after each dose of vaccination;]
Serious Adverse Events (SAE)[Within 12 months from first vaccination to completion of full vaccination schedule;]
Level of SARS-CoV-2 IgG antibody (tested by ELISA)[Day 28 post full vaccination;]
The seropositive rates and level of SARS-CoV-2 neutralizing antibody and IgG
Immunogenicity index-Seroconversion rate of the neutralizing antibody [Time Frame: Day 14 after the full course vaccination]
Neutralizing antibody assay will be performed using the micro-neutralization method. Seroconversion will be defined as a change from seronegative(<1:8) to seropositive(≥1:8), or ≥4 fold increase from baseline

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SARS-CoV-2 serum neutralizing titers - SCR [Time Frame: 1 Week, 6 and 12 Months after Dose 2]
Compared with baseline before Vaccination 1, SCR of SARS-CoV-2 serum neutralizing titers at 1 week, 6 and 12 months after dose 2.
SARS-CoV-2 serum neutralizing titers - GMT [Time Frame: 1 Week, 6 and 12 Months
The proportion of adverse events (AE) within 7 days after each dose of the recombinant SARS-CoV-2 vaccine (CHO cell) or placebo. [Time Frame: 7 days after each dose]
The proportion of abnormal markers of hematology, blood chemistry and urine analysis within 3 days after each dose of the recombinant SARS-CoV-2 vaccine (CHO
The incidence of adverse events (AE) [Time Frame: 0 to 28 days after vaccination in each study group]
The incidence of adverse events (AE)
The incidence of severe adverse events (SAE) [Time Frame: 0 to 28 days after vaccination in each study group]

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Time of disappearance of fever symptoms [Time Frame: From date of randomization until the date of discharge, assessed up to 6 months]
Time of disappearance of fever in degrees centigrade
The rate of conversion from COVID-19 positive to COVID-19 negative [Time Frame: From date of randomization until the date of discharge, assessed up to 6 months]

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Proportion of subjects with infusion-related reactions [Time Frame: up to 24 weeks]
Proportion of subjects with hypersensitivity reactions [Time Frame: up to 24 weeks]
Mean of change from pre-dose baseline in ECG readings [Time Frame: up to 24 weeks]
ECG readings include PR interval, RR interval, QRS interval, etc.

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The incidence of adverse events (AE) [Time Frame: from day 0 to day 30 after last dose]
The incidence of adverse events (AE)
The incidence of grade 3 adverse events (AE) [Time Frame: from day 0 to day 30 after last dose]

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Duration of COVID-19 related symptoms through Day 29 among subjects with mild-moderate COVID-19 [Time Frame: up to Day 29]
Proportion of subjects who have mild-moderate COVID-19 and develop severe COVID-19 after randomization [Time Frame: up to 26 weeks]
Assessment of PK parameters (Group A and B): maximum serum concentration

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Safety index-incidence of adverse reactions within 56 days after the first dose vaccination [Time Frame: Day 0-56 after the first dose vaccination]
Incidence of adverse reactions within 56 days after the first dose vaccination
Safety index-incidence of serious adverse events [Time Frame: Day 0-56 after the first dose vaccination]

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The incidence of adverse events (AE) [Time Frame: 0 to 30 days after vaccination in each study group]
The incidence of severe adverse events (SAE) [Time Frame: 12 months after prime and boost vaccination]
The Geometric mean titer (GMT) of anti-SARS-CoV-2 specific neutralizing antibody

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Incidence of adverse reactions within 28 days after vaccination. [Time Frame: Within 28 days after vaccination]
Incidence of adverse reactions within 28 days after vaccination.
Incidence of adverse events within 28 days after vaccination. [Time Frame: Within 28 days after vaccination]

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Incidence of Adverse Events/Reactions (AE/AR) [Time Frame: Within 0-7 days after each vaccination]
The occurence of Adverse Events/Reactions (AE/AR) in all groups
Incidence of Adverse Events/Reactions (AE/AR) [Time Frame: Within 0-28 days after each vaccination]
Geometric Mean Titer (GMT) of SARS-CoV-2 Specific Neutralizing Antibody (Live SARS-CoV-2 neutralization assay) On the 30th day after the full course of vaccination, the neutralizing antibody level (wild Strains) [Time Frame: 30th day after the full course of vaccination]
Phase 1
The Geometric Mean Titer (GMT) of anti-SARS-CoV-2 neutralizing antibody [Time Frame: 14 days after the second dose in schedule of Day 0,21,42]
Neutralizing antibody assay will be performed using the Microcytopathic assay
The four-fold increase rate of anti-SARS-CoV-2 neutralizing antibody [Time Frame: 14 days after the second dose in schedule of Day 0,21,42]

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Safety index-incidence of adverse reactions [Time Frame: Day 0-7 after each dose vaccination]
Incidence rate of adverse reactions within 7 days after each dose
Safety index-incidence of serious adverse events [Time Frame: From the beginning of the vaccination to 6 months after the second dose vaccination]

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the death in hospital [Time Frame: before and 48 hours after intervention]
the rate of death in hospital (%)
the composite endpoint of aggravation [Time Frame: before and 48 hours after intervention]
the rate of composite endpoint of aggravation(%) according to upgraded oxygen
Seroconversion rate of SARS-CoV-2 neutralizing antibody [Time Frame: 28 days after vaccination]
Seroconversion rate of anti SARS-CoV-2 specific neutralizing antibody (wild type variants test).
Incidence of solicited AE within 14 days after the booster dose [Time Frame: within 14 days after the booster dose]
Incidence of solicited adverse events (AE) within 14 days after the booster vaccination.

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Nucleic acid negative time
The ratio of nucleic acid to negative [Day 7/14]
Major symptoms disappeared [Day 7/14]
CT imaging changes of chest [Day 7/14]
Hospital stays

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Assess Pharmacokinetic parameters of LY-CovMab [Time Frame: up to 99 days]
Maximum observed concentration (Cmax)

Assess Pharmacokinetic parameters of LY-CovMab [Time Frame: up to 99 days]
Time to maximum observed concentration (Tmax)

Seroconversion rate of the neutralizing antibody to SARS-CoV-2 [Time Frame: Day 4, 14, 32,42 and 56 after the first dose]
Seroconversion rate of the neutralizing antibody to SARS-CoV-2 at different points after vaccination

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Incidence of serious AEs (SAEs) and adverse events of special interest (AESIs) [Time Frame: Day 0 to Month 13]
Percentage of participants with SAEs or AESI for 12month after last dose vaccination

Proportion of subjects with abnormal markers of hematology, biochemistry, urinalysis
Percentage of participants reporting adverse events (AEs) [Time Frame: From dose 1 through 28 days after the last dose]
Percentage of participants with solicited AEs (local, systemic) for 28 days following each primary vaccination (Days 0, 28) by intensity, relevance.

Immunogenicity index of seroconversion rate of the neutralizing antibody [Time Frame: Day 28 after the whole vaccination of COVID-19 vaccine]
Seroconversion rate of the neutralizing antibody to live SARS-CoV-2 at day 28 after the whole vaccination of COVID-19 vaccine

Incidence of adverse reactions/events [Time Frame: 0-28 days after each vaccination]
Serious Adverse Events (SAE) [Time Frame: within 12 months post full primary immunization course and the booster dose respectively]

Seroconversion rate of SARS-CoV-2 IgG binding antibody [Time Frame: 28 days
Incidence of Serious Adverse Events (SAE) [Time Frame: within 12 months post full vaccination]
Incidence of abnormal indicators of laboratory safety examinations(including blood routine, blood biochemistry, urine routine, thyroid function and coagulation function) [Time Frame: Day 3 after each dose of vaccination]

Seroconversion rate of the neutralizing antibody to live SARS-CoV-2 in total population [Time Frame: Day 28 after the second dose]
Seroconversion rate of the neutralizing antibody to live SARS-CoV-2 at day 28 after the second dose in total population

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Phase I/II clinical trial：The incidence of adverse events (AEs) [Time Frame: Day 0-7 days after each vaccination.]
Adverse events (AEs) in 0-7 days after each vaccination.

Phase I/II clinical trial：The incidence of adverse events (AEs) in all participants. [

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Virus negative conversion rate [Time Frame: 14-20 days]
virus negative conversion rate in second week, overall virus negative conversion rate

Antipyretic rate [Time Frame: 14-20 days]
defined as: the rate of Axillary temperature <37.5 ℃ for at least 48h

Changes in the patients' clinical status through the WHO Clinical Progression Scale (scores 0-10) at day 7 [Time Frame: Day 7]
The scale provides a measure of illness severity across a range from 0 (not infected) to 10 (dead)

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Incidence of adverse events within 0-28 days after the booster dose. [Time Frame: within 28 days after the booster dose.]
Incidence of adverse events (AE) within 0-28 days after the booster vaccination.

Incidence of serious adverse events (SAE) till the 12 months after the booster dose. [Time Frame: within 12 months after the booster dose.]

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Incidence of solicited AE within 14 days after the booster dose [Time Frame: within 14 days after the booster dose]
Incidence of solicited adverse events (AE) within 14 days after the booster vaccination.

Incidence of unsolicited AE within 28 days after the booster dose. [Time Frame: Safety index-incidence of serious adverse events [Time Frame: 1 month after booster immunization]
Incidence rate of serious adverse events within 1 month after booster immunization of the Inactivated SARS-CoV-2 vaccine

Adverse events following vaccination [Time Frame: 0-6 months]
analyse the incidence of adverse events following immunization, both solicited and unsolicited

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All cause mortality ascertained from data analysed to day 28 [Time Frame: At 28 days from inclusion]
All cause mortality ascertained from data analysed to day 28
Ventilator-free days within 28 days [Time Frame: At 28 days from inclusion]
Ventilator-free days within 28 days
Immunogenicity index-seropositive rate of neutralizing antibodies (CZ02 strain) [Time Frame: Day 28 after booster vaccination]
The seropositive rate of neutralizing antibodies (CZ02 strain) 28 days after booster vaccination

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Antibody IgM, IgG levels [3 days, 7 days, 10 days, and 14 days after vaccination injection, and 14 days and 28 days after discharge]
The neutralizing antibody S, N, total antibody [14days, 28 days after vaccine injection, and 14 and 28 days after discharge]
Memory B cell ratio

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Secondary endpoint: [Time Frame: 14 days after vaccination]
SARS-cov-2 neutralizing antibody GMT on day 14 after full immunization with recombinant Novel Coronavirus vaccine (CHO cells);

Secondary endpoint: [Time Frame: 1 months after full vaccination]

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Immunogenicity index-Seropositivity rate of the neutralizing antibody to live SARS-CoV-2 [Time Frame: Day 28 after the second dose of COVID-19 vaccine]
Seropositivity rate of the neutralizing antibody to live SARS-CoV-2 at day 28 after the second dose of COVID-19 vaccine.

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Geometric mean increase (GMI) of the neutralizing antibody to live SARS-CoV-2 [Time Frame: 28 days after the full immunization]
Geometric mean increase (GMI) of the neutralizing antibody to live SARS-CoV-2 28 days after the full immunization of each lot of COVID-19 vaccine
Seroconversion rate of the neutralizing antibody to live SARS-CoV-2 [Time Frame:
Incidence of solicited adverse events within 7 days after each dose. [Time Frame: within 7 days after each dose]
Incidence of solicited adverse events within 7 days after each dose.

Incidence of adverse events within 28 days after each dose. [Time Frame: within 28
Safety index-Incidence of adverse reactions [Time Frame: 0-28 days after booster]
Incidence of adverse reactions occured 0-28 days after booster

Safety index-Incidence of adverse reactions [Time Frame: 0-7 days after booster dose]
Immunogenicity index-Seropositivity rate of neutralizing antibodies(CZ02 strain) [Time Frame: 14 days after the booster dose vaccination]
Seropositivity rate of neutralizing antibodies(CZ02 strain) 14 days after the booster dose vaccination
maximum concentration (Cmax) [Time Frame: 29 days after the last participant is randomized]
PK parameters to be evaluated for IBI314 including maximum concentration (Cmax) will be determined when appropriate.
area under the concentration-time curve (AUC) [Time Frame: 29 days after the last

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Pharmacokinetics: AUC [Time Frame: Day 1 to D85]
Area under the plasma concentration-time curve from the time of dosing to the last measurable concentration time t (AUC0-last)
Pharmacokinetics: Tmax [Time Frame: Day 1 to D85]
Time to maximum concentration (Tmax)
IgG antibody concentration [Time Frame: 8 weeks]
The IgG antibody concentration of S protein and RBD in SARS-CoV-2 serum.

T-cell response level [Time Frame: 8 weeks]
Test of T-cell response level by spot-forming cell(SFC) counting, using Champspot III

IgG antibody concentration [Time Frame: 8 weeks]
The IgG antibody concentration of S protein and RBD in SARS-CoV-2 serum.

T-cell response level [Time Frame: 8 weeks]
Test of T-cell response level by spot-forming cell(SFC) counting, using Champspot III

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Incidence of adverse reactions within 30 minutes after the booster dose. [Time Frame: Within 30 minutes after the booster dose]
Incidence of adverse reactions within 30 minutes after the booster dose.
Incidence of adverse reactions within 14 days after the booster dose. [Time Frame: Within 14 days after the booster dose]

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Pharmacokinetic parameters(Cmax) [Time Frame: 0-Day 5]
Estimate of the Cmax for single and multiple dose administration of FB2001.

Pharmacokinetic parameters(AUC0-τ) [Time Frame: 0-Day 5]
Estimate of AUC0-τ for single and multiple dose administration of FB2001

The seropositive rate of neutralizing antibodies [Time Frame: 3 days/14 days/28days/45days/60 days/90 days/180 days/365 days]
The seropositive rate of neutralizing antibodies after third vaccination shots

GMT of neutralizing antibodies [Time Frame: 3 days/14 days/28days/45days/60

Phase I: GMT and seroconversion rate of total IgG antibody (ELISA method) against the Alpha, Beta, Delta variants SARS-CoV-2 and T4-Foldon on Day 28 after the first dose of vaccination; [Time Frame: Day 28 after the first dose of vaccination]
GMT and seroconversion rate of total IgG antibody against the Alpha, Beta, Delta variants of SARS-CoV-2 and T4-Foldon on Day 28 after the first dose of vaccination

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A randomized, controlled open-label trial to evaluate the efficacy and safety of lopinavir-ritonavir in hospitalized patients with 2019-nCoV infection
No Statistically Apparent Difference in Antiviral Effectiveness Observed Among Ribavirin Plus Interferon-Alpha, Lopinavir/Ritonavir Plus Interferon-Alpha, and Ribavirin Plus
Comparative effectiveness and safety of ribavirin plus interferon-alpha, lopinavir/ritonavir plus interferon-alpha and ribavirin plus lopinavir/ritonavir plus interferon-alphain in patients with mild to moderate novel coronavirus pneumonia
Guideline-Based Chinese Herbal Medicine Treatment Plus Standard Care for Severe Coronavirus Disease 2019 (G-CHAMPS): Evidence From China
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SARS-CoV-2 clearance in COVID-19 patients with Novaferon treatment: A randomized, open-label, parallel-group trial
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Efficacy and safety of Reduning injection in the treatment of COVID-19: a randomized, multicenter clinical study
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High-flow nasal-oxygenation-assisted fibreoptic tracheal intubation in critically ill patients with COVID-19 pneumonia: a prospective randomised controlled trial

Nasal high-fow preoxygenation assisted fibre-optic bronchoscope intubation in patients with critical novel coronavirus pneumonia (COVID-19): a randomized clinical trial

Effect of Convalescent Plasma Therapy on Time to Clinical Improvement in Patients With Severe and Life-threatening COVID-19: A Randomized Clinical Trial

Convalescent plasma for the treatment of severe and critical novel coronavirus pneumonia (COVID-19): a prospective randomized controlled trial

A Randomized, Open-Label, Controlled Clinical Trial of Azvudine Tablets in the Treatment of Mild and Common COVID-19, a Pilot Study

A randomized, open-label, controlled clinical trial for azvudine in the treatment of novel coronavirus pneumonia (COVID-19)

Hydroxychloroquine in patients with mainly mild to moderate coronavirus disease 2019: open label, randomised controlled trial

Hydroxychloroquine treating novel coronavirus pneumonia (COVID-19): a randomized controlled, open label, multicenter trial

Transplantation of ACE2- Mesenchymal Stem Cells Improves the Outcome of Patients with COVID-19 Pneumonia

Mesenchymal stem cell treatment improves outcome of COVID-19 patients via multiple immunomodulatory mechanisms

Clinical trials of mesenchymal stem cells for the treatment of pneumonitis caused by novel coronavirus (COVID-19)

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Mycobacterium vaccae Nebulization in the Treatment of COVID-19: A Randomized, Double-Blind, Placebo-Controlled Trial

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Potential therapeutic effects of dipyridamole in the severely ill patients with COVID-19

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A multicenter, randomized, double-blind, controlled clinical trial for leflunomide in the treatment of novel coronavirus pneumonia (COVID-19)

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Safety and immunogenicity of an inactivated COVID-19 vaccine, BBIBP-CorV, in people younger than 18 years: a randomised, double-blind, controlled, phase 1/2 trial

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Safety and immunogenicity of the SARS-CoV-2 BNT162b1 mRNA vaccine in younger and older Chinese adults: a randomized, placebo-controlled, double-blind phase 1 study

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Immunogenicity and safety of a severe acute respiratory syndrome coronavirus 2 inactivated vaccine in healthy adults: randomized, double-blind, and placebo-controlled phase 1 and phase 2 clinical trials

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Evaluation of the safety and immunogenicity of inactivated SARS-CoV-2 Vaccine(Vero Cells) in healthy population aged 18 years and above: a randomized, double-blind, placebo parallel-controlled phase I clinical trial

Evaluation of the safety and immunogenicity of inactivated SARS-CoV-2 Vaccine (Vero Cells) in healthy population aged 18 years and above: a randomized, double-blind, placebo parallel-controlled phase II clinical trial

Immunogenicity and Safety of a Three-Dose Regimen of a SARS-CoV-2 Inactivated Vaccine in Adults: A Randomized, Double-blind, Placebo-controlled Phase 2 Trial

Evaluation of the safety and immunogenicity of inactivated SARS-CoV-2 Vaccine (Vero Cells) in healthy population aged 18 years and above: a randomized, double-blind, placebo parallel-controlled phase II clinical trial

The Effect of Cognitive Behavioral Therapy on Depression, Anxiety, and Stress in Patients With COVID-19: A Randomized Controlled Trial

The effect of cognitive behavioural therapy on depression, anxiety and stress in patients with COVID-19: a randomized controlled trial

Exploring an Integrative Therapy for Treating COVID-19: A Randomized Controlled Trial

Effects of Traditional Chinese Medicines (TCMs) on Patients With COVID-19 Infection: A Perspective, Open-labeled, Randomized, Controlled Trial

Antiviral Activity and Safety of Darunavir/Cobicistat for the Treatment of COVID-19

Efficacy and Safety of Darunavir and Cobicistat for Treatment of COVID-19

Efficacy and safety of lopinavir/ritonavir or arbidol in adult patients with mild/moderate COVID-19: an exploratory randomized controlled trial

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A Randomized, Open-label, Controlled Study of the Efficacy of Lopinavir Plus Ritonavir and Arbidol for Treating With Patients With Novel Coronavirus Infection

Remdesivir in adults with severe COVID-19: a randomised, double-blind, placebo-controlled, multicentre trial

A Phase 3 Randomized, Double-blind, Placebo-controlled, Multicenter Study to Evaluate the Efficacy and Safety of Remdesivir in Hospitalized Adult Patients With Severe COVID-19.

硫酸羟氯喹治疗2019冠状病毒病(COVID-19)普通型患者的初步研究

Efficacy and Safety of Hydroxychloroquine for Treatment of COVID-19

Pilot trial of high-dose vitamin C in critically ill COVID-19 patients

Vitamin C Infusion for the Treatment of Severe 2019-nCoV Infected Pneumonia: a Prospective Randomized Clinical Trial

Early Use of Corticosteroid May Prolong SARS-CoV-2 Shedding in Non-Intensive Care Unit Patients with COVID-19 Pneumonia: A Multicenter, Single-Blind, Randomized Control Trial

Efficacy and Safety of Corticosteroids in COVID-19: A Prospective Randomized Controlled Trails

Bromhexine Hydrochloride Tablets for the Treatment of Moderate COVID-19: An Open-Label Randomized Controlled Pilot Study

Evaluating the Efficacy and Safety of Bromhexine Hydrochloride Tablets Combined With Standard Treatment/ Standard Treatment in Patients With Suspected and Mild Novel Coronavirus Pneumonia (COVID-19)

Effect of human umbilical cord-derived mesenchymal stem cells on lung damage in severe COVID-19 patients: a randomized, double-blind, placebo-controlled phase 2 trial

Human mesenchymal stem cells treatment for severe COVID-19: 1-year follow-up results of a randomized, double-blind, placebo-controlled trial

A Phase II, Multicenter, Randomized, Double-blind, Placebo-controlled Trial to Evaluate the Efficacy and Safety of Human Umbilical Cord-derived Mesenchymal Stem Cells in the Treatment of Severe COVID-19 Patients

Immunogenicity and safety of a recombinant adenovirus type-5-vectored COVID-19 vaccine in healthy adults aged 18 years or older: a randomised, double-blind, placebo-controlled, phase 2 trial

A Randomized, Double-blind, Placebo-controlled Phase II Clinical Trial to Evaluate the Safety and Immunogenicity of the Recombinant Novel Coronavirus Vaccine (Adenovirus Vector) in Healthy Adults Aged Above 18 Years

Safety, tolerability, and immunogenicity of an inactivated SARS-CoV-2 vaccine in healthy adults aged 18–59 years: a randomised, double-blind, placebo-controlled, phase 1/2 clinical trial

A Randomized, Double-Blinded, Placebo-Controlled, Phase I/II Clinical Trial, to Evaluate the Safety and Immunogenicity of the SARS-CoV-2 Inactivated Vaccine in Healthy Adults Aged 18~59 Years

Safety, tolerability, and immunogenicity of an inactivated SARS-CoV-2 vaccine (CoronaVac) in healthy adults aged 60 years and older: a randomised, double-blind, placebo-controlled, phase 1/2 clinical trial

A Randomized, Double-blind, Placebo-controlled Clinical Trial, to Evaluate Safety and Immunogenicity of Inactivated SARS-CoV-2 Vaccine (Vero Cell), in Healthy Elderly Aged 60 Years and Above

Immunogenicity and safety of a third dose of CoronaVac, and immune persistence of a two-dose schedule, in healthy adults: interim results from two single-centre, double-blind, randomised, placebo-controlled phase 2 clinical trials

A Randomized, Double-Blinded, Placebo-Controlled, Phase I/II Clinical Trial, to Evaluate the Safety and Immunogenicity of the SARS-CoV-2 Inactivated Vaccine in Healthy Adults Aged 18~59 Years

A Randomized, Double-blind, Placebo-controlled Clinical Trial, to Evaluate Safety and Immunogenicity of Inactivated SARS-CoV-2 Vaccine (Vero Cell), in Healthy Elderly Aged 60 Years and Above

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Randomized, double-blinded and placebo-controlled phase II trial of an inactivated SARS-CoV-2 vaccine in healthy adults

The safety and immunogenicity of an inactivated SARS-CoV-2 vaccine in Chinese adults aged 18–59 years: A phase I randomized, double-blinded, controlled trial

A Randomized, Double-blind, Placebo-controlled, Phase Ia/IIa Trial of an Inactivated SARS-CoV-2 Vaccine in Healthy People Aged 18 to 59 Years

Tolerability, Safety, Pharmacokinetics, and Immunogenicity of a Novel SARS-CoV-2 Neutralizing Antibody, Etesevimab in Chinese Healthy Adults: A Randomized, Double-Blind, Placebo-Controlled, First-In-Human Phase 1 Study

A Randomized, Double-blind, Placebo-controlled, Phase I Clinical Study to Evaluate the Tolerability, Safety, Pharmacokinetic Profile and Immunogenicity of JS016 (Anti-SARS-CoV-2 Monoclonal Antibody) Injection in Chinese Healthy Subjects After Intravenous Infusion of Single Dose

Safety and immunogenicity of a recombinant tandem-repeat dimeric RBD-based protein subunit vaccine (ZF2001) against COVID-19 in adults: two randomised, double-blind, placebo-controlled, phase 1 and 2 trials

A Multi-center, Double-blind, Randomized, Placebo Parallel Controlled, Safety and Tolerability Phase I Clinical Trial of Recombinant Novel Coronavirus Vaccine (CHO Cells) in Healthy People Between 18 and 59 Years of Age

A Randomized, Blinded, Placebo-controlled Trial to Evaluate the Immunogenicity and Safety of a Recombinant New Coronavirus Vaccine (CHO Cell) With Different Doses and Different Immunization Procedures in Healthy People Aged 18 to 59 Years

Safety and immunogenicity of a recombinant COVID-19 vaccine (Sf9 cells) in healthy population aged 18 years or older: two single-center, randomised, double-blind, placebocontrolled, phase 1 and phase 2 trials

Safety, Tolerability and Immunogenicity of a Recombinant SARS-CoV-2 Vaccine (Sf9 Cell) in Chinese Healthy Population Aged 18 Years and Older: A Phase I, Single-center, Randomized, Placebo-controlled, Double-blind Study

A Single-center, Randomized, Double-Blinded, Placebo-Controlled, Phase II Clinical Trial of Recombinant COVID-19 Vaccine (Sf9 Cells), in the Subjects From Healthy Aged 18 Years and Above

Clinical Outcomes and Plasma Concentrations of Baloxavir Marboxil and Favipiravir in COVID-19 Patients: An Exploratory Randomized, Controlled Trial

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A randomized controlled trial for the efficacy and safety of Baloxavir Marboxil, Favipiravir tablets in novel coronavirus pneumonia (COVID-19) patients who are still positive on virus detection under the current antiviral therapy

Efficacy of Jinhua Qinggan Granules Combined With Western Medicine in the Treatment of Confirmed and Suspected COVID-19: A Randomized Controlled Trial

Efficacy of Huoxiang Zhengqi dropping pills and Lianhua Qingwen granules in treatment of COVID-19: A randomized controlled trial

Community based prevention and control for Chinese medicine in the treatment of novel coronavirus pneumonia (COVID-19) in the isolated population

Chinese Medicine Formula Huashibaidu Granule Early Treatment for Mild COVID-19 Patients: An Unblinded, Cluster-Randomized Clinical Trial

Study on Prevention and Treatment of Novel Coronavirus Pneumonia (COVID-19) With Traditional Chinese Medicine

Traditional Chinese medicine shenhuang granule in patients with severe/critical COVID-19: A randomized controlled multicenter trial

A multicenter, randomized, controlled trial for integrated chinese and western medicine in the treatment of novel coronavirus pneumonia (COVID-19) based on the 'Truncated Torsion' strategy

A Randomized, Single-Blind, Group Sequential, Active-Controlled Study to Evaluate the Clinical Efficacy and Safety of α-Lipoic Acid for Critically Ill Patients With Coronavirus Disease 2019 (COVID-19)

A multicenter, randomized controlled trial for the efficacy and safety of Alpha lipoic acid (iv) in the treatment of patients of severe novel coronavirus pneumonia (COVID-19)

Evaluation of the efficacy and safety of hydroxychloroquine in comparison with chloroquine in moderate and severe patients with COVID-19

Evaluation the Efficacy and Safety of Hydroxychloroquine Sulfate in Comparison with Phosphate Chloroquine in Severe Patients with Novel Coronavirus Pneumonia (COVID-19): a Randomized, Open-Label, Parallel, Controlled Trial

For peer review only

Evaluation the Efficacy and Safety of Hydroxychloroquine Sulfate in Comparison with Phosphate Chloroquine in Mild and Commen Patients with Novel Coronavirus Pneumonia (COVID-19): a Randomized, Open-label, Parallel, Controlled Trial

Effect of qigong exercise and acupressure rehabilitation program on pulmonary function and respiratory symptoms in patients hospitalized with severe COVID-19: a randomized controlled trial

Liu-Zi-Jue Qigong and Acupressure Therapy for Pulmonary Function and Quality of Life in Patient with Severe novel coronavirus pneumonia (COVID-19): A Randomized Controlled Trial

Clinical efficacy of low-dose emetine for patients with COVID-19: a real-world study

A parallel, randomized controlled clinical trial for the efficacy and safety of Pediatric Huatanzhike granules (containing ipecacuanha tincture) in the treatment of mild and moderate novel coronavirus pneumonia (COVID-19)

Combination of Hua Shi Bai Du granule (Q-14) and standard care in the treatment of patients with coronavirus disease 2019 (COVID-19): A single-center, open-label, randomized controlled trial

Study on Prevention and Treatment of Novel Coronavirus Pneumonia (COVID-19) With Traditional Chinese Medicine

Negative-Pressure Isolation Mask for Endoscopic Examination During the Coronavirus Disease 2019 Pandemic: A Randomized Controlled Trial

A prospective randomized controlled trial for a Self-made Gastroscope isolation mask for preventing and controlling the novel coronavirus pneumonia (COVID-19) Epidemic period

六神丸联合常规治疗对 COVID-19 患者的临床疗效

A randomized parallel controlled trial for LIUSHENWAN in Treatment of Novel Coronavirus Pneumonia (COVID-19)

Chansu improves the respiratory function of severe COVID-19 patients

Observation Of Clinical Efficacy And Safety Of Bufonis Venenum Injection In The Treatment Of Severe Novel Coronavirus Pneumonia (COVID-19)

Therapeutic effects of traditional Chinese medicine (Maxingshigan-Weijing Decoction) on COVID-19: An open-label randomized controlled trial

Study for the therapeutic effect and mechanism of traditional Chinese medicine in the treatment of novel coronavirus pneumonia (COVID-19)

A telerehabilitation programme in post-discharge COVID-19 patients (TERECO): a randomised controlled trial

A clinical study for the effect of remote monitoring exercise rehabilitation on the discharged patients with novel coronavirus pneumonia (COVID-19)

Efficacy and safety of Bufei Huoxue capsules in the management of convalescent patients with COVID-19 infection: A multicentre, double-blind, and randomised controlled trial

A randomized, double-blind, controlled trial for Bufei Huoxue Capsule in the treatment of convalescent COVID-19 patients

Safety and immunogenicity of the SARS-CoV-2 ARCoV mRNA vaccine in Chinese adults: a randomised, double-blind, placebo-controlled, phase 1 trial

A Phase Ib clinical trial to evaluate the safety, tolerance and preliminary immunogenicity of different doses of a SARS-CoV-2 mRNA vaccine in population aged 18-59 years

The effects of narrative exposure therapy on COVID-19 patients with post-traumatic stress symptoms: A randomized controlled trial

The Effects of Narrative Exposure Therapy on COVID-19 Patients with Post-Traumatic Stress Disorder: a Randomized Controlled Trial

Safety and immunogenicity of inactivated SARS-CoV-2 vaccine in high-risk occupational population: a randomized, parallel, controlled clinical trial

A randomized, controlled, multicenter study on the safety and immunogenicity of SARS-CoV-2 inactivated vaccine

A randomized, controlled, multicenter study on the safety and immunogenicity of SARS-CoV-2 inactivated vaccine

Efficacy and Safety of Lianhuaqingwen Capsules for the Prevention of Coronavirus Disease 2019: A Prospective Open-Label Controlled Trial

Real-world study on the prevention effect of Lian-Hua Qing-Wen Capsules on close contacts of new coronavirus pneumonia

Immunogenicity and safety of a recombinant fusion protein vaccine (V-01) against coronavirus disease 2019 in healthy adults: a randomized, double-blind, placebo-controlled, phase II trial

A Randomized, Double-blind, Placebo-controlled Phase II Clinical Trial to Evaluate the Immunogenicity and Safety of Recombinant SARS-CoV-2 Fusion Protein Vaccine (V-01) in Healthy Subjects

Safety and immunogenicity of a recombinant interferon-armed RBD dimer vaccine (V-01) for COVID-19 in healthy adults: a randomized, double-blind, placebo-controlled, Phase I trial

A Randomized, Double-blind, Placebo-controlled Phase I Clinical Trial to Evaluate the Safety and Immunogenicity of Recombinant SARS-CoV-2 Fusion Protein Vaccine (V-01) in Healthy Subjects

Favipiravir in the treatment of patients with SARS-CoV-2 RNA recurrent positive after discharge: A multicenter, open-label, randomized trial

The Mechanism, Clinical Outcome and Therapeutic Intervention of Corona Virus Disease 2019 Patients Whose Nucleic Acids Changed From Negative to Positive

Safety, Tolerability, Pharmacokinetics, and Immunogenicity of a Monoclonal Antibody (SCTA01) Targeting SARS-CoV-2 in Healthy Adults: a Randomized, Double-Blind, PlaceboControlled, Phase I Study

A Randomized, Double-blinded, Placebo-controlled, Single Ascending Dose, Phase I Study to Evaluate the Tolerability, Safety, Pharmacokinetics of SCTA01 in Healthy Subjects

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Safety, tolerability, pharmacokinetic characteristics, and immunogenicity of MW33: a Phase 1 clinical study of the SARS-CoV-2 RBD-targeting monoclonal antibody

A Phase I Clinical Study to Evaluate the Safety, Tolerability, Pharmacokinetic Characteristics and Immunogenicity of a Single Dose of MW33 Injection in Healthy Subjects

A Multicenter, Randomized, Double-Blind, Placebo-Controlled Phase 2 Clinical Trial of MW33 Injection to Evaluate the Efficacy and Safety in Patients With Mild or Moderate COVID-19

Safety, tolerability, and immunogenicity of an inactivated SARS-CoV-2 vaccine (CoronaVac) in healthy children and adolescents: a double-blind, randomised, controlled, phase 1/2 clinical trial

A Randomized, Double-Blinded, Placebo-Controlled, Phase I/II Clinical Trial, to Evaluate the Safety and Immunogenicity of the SARS-CoV-2 Inactivated Vaccine (Vero Cell) in Healthy Population Aged 3-17 Years

Safety and immunogenicity of a recombinant adenovirus type-5-vectored COVID-19 vaccine with a homologous prime-boost regimen in healthy participants aged 6 years and above: a randomised, double-blind, placebo-controlled, phase 2b trial

A Randomized, Double-blind, Placebo -Controlled Phase IIb Clinical Trial to Evaluate the Safety and Immunogenicity of Ad5-nCoV in Person 6 Years of Age and Older and Those Who Have Previously Been Vaccinated With Ad5-EBOV

Heterologous AD5-nCOV plus CoronaVac versus homologous CoronaVac vaccination: a randomized phase 4 trial

Safety and Immunogenicity of Sequential Immunization of Inactivated SARS-CoV-2 Vaccine and Recombinant SARS-CoV-2 Vaccine (Ad5 Vector) in Chinese Healthy Adults Aged 18-59 Years: a Randomized, Observer-blind, Parallel-controlled Clinical Trial

A Randomized, Double-Blind, Placebo-Controlled, First-in-Human Clinical Trial to Assess Safety, Tolerability, and Pharmacokinetics of LY-CovMab, a Potent Human Neutralizing Antibody Against SARSCoV-2

A Phase I, Single Ascending Dose Study to Evaluate the Safety, Tolerability, Pharmacokinetics, and Immunogenicity of LY-CovMab Injection in Chinese Healthy Subjects

Safety and immunogenicity of a third-dose homologous BBIBP-CorV boosting vaccination: interim results from a prospective open-label study

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Reactogenicity and Immunogenicity of Third Dose Vaccine Booster Following Two Doses of Inactivated Vaccines

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Deviation in Estimated Enrollment (1 if Yes, 0 if not specified in one or more of data sources)	Absoulute Deviation by Precentage	Actual Enrollment (Participants Randomized)/Original Estimated Enrollment in Registry
1	24.40%	199 (The planned enrollment of 160 patients in the trial occurred quickly, 160
1	6.50%	101 (the efficient control and suppression of the COVID-19 outbreak in 108
		42 42
1	1.10%	89 90
1	30.80%	157 120
1	4%	96

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		100
		60
		60
1	48.50%	103 (The last patient enrolled in this study was on March 27, 2020, and for the next 3 days, we were
		200
		20
		20
		360
		360
1	91.70%	10
1	51.70%	58
		120

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		200
		200
1	48.30%	31
		60
1	93.30%	31
		460
1	95%	10
1	75%	50
		200
1	51.70%	273
		180
		60

		60
1	82.70%	26
		150
1	78%	96 (phase 1) and 224 (phase 2)
1	23.10%	784 (Phase I), 336 (Phase II)
		1456
1	69.90%	phase I: 192, phase II: 448
1	52.60%	288 (Phase I), 720 (Phase II)
		2128
		144
		144
1	52.50%	phase I: 60, phase II: 500

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		180
		1000
1	50%	500
		1000
		93
		93
1	68%	48
		150
		30
		30
1	31.20%	86 (Based on the estimated number of patients admitted to the hospital at that time, we initially

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		125
1	47.60%	237
		452
		30
		30
1	60%	56 (With the control of the epidemic, this trial was stopped early, and the number of qualifying
		140
1	78.50%	86 (Because the number of patients with COVID-19 pneumonia in all the centers decreased in late
		400
1	70%	18 (The number of patients finally included was less than expected due to the successful containment of
		60

1	124.40%	101
1	124.40%	101
		45
1	1.60%	508
		500
		744
		744
		422
		422
1	27.70%	18–59 years: 540 60 years and older: 303
		744
		422

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	750
	192
	942
	40
	40
	50 (phase I), 900 (phase II)
	50
	900
	168 (Phase I), 960 (Phase II)
	168
	960
	30

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		30
1	69.30%	123
1	29.30%	283
		400
		408
		408
1	23.80%	122
		160
1	75%	17
		68
1	56%	88
		100

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		100
1	36%	128
		200
1	61%	39
		100
		204
		204
1	6.70%	320
		300
1	16.70%	80
		96
		50

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		50
1	15.70%	59
		70
		120
		120
1	9.20%	131
		120
		120
		120
		111
		111
1	12.40%	809

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		360
		360
1	10.20%	1976
		2200
		880
		880
		180
		180
1	73.80%	55
		210
		33
		33

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1	76.90%	42
		32
		150
		72 (Phase I), 480 (Phase II)
		552
1	10.60%	430
		481
		300
		300
		42
		42
1	74.30%	103

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400

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Deviation in Center (1 if Yes, 0 if not specified in one or more of data sources)	Center	Deviation in Inclusion Criteria (1 if Yes, 0 if not specified in one or more of data sources)
	Jin Yin-Tan Hospital Wuhan Jinyintan Hospital (Wuhan Infectious Diseases Hospital)	1
	Chongqing Public Health Medical Center Chongqing Public Health Medical Center	1
	Hubei Provincial Hospital of Integrated Chinese and Western Medicine Hubei Provincial Hospital of Integrated Chinese and Western Medicine	
1	First Hospital of Changsha The First Hospital of Changsha The Second Xiangya Hospital of Cental South University	1
0	Not specified Hubei Integrated Traditional Chinese and Western Medicine Hospital Huanggang Central Hospital (Dabieshan Medical Center)	1
0	Not specified	

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	West China Hospital, Sichuan University Chengdu Public Health Clinical Medical Center Wuhan Red Cross Hospital	
0	Not specified	1
	The Fourth People's Hospital of Wuhan	
1	Union Hospital of Tongji Medical College of Huazhong University of Science and Technology; Union Hospital, Tongji Medical College, Huazhong university of Science and Technology Red Cross Hospital in Wuhan	
	Guangshan County People's Hospital People's Hospital of Guangshan County	
0	Not specified	1
	Suizhou Central Hospital Xiaogan Central Hospital Wuhan 7th Hospital Ezhou Central Hospital Jingzhou First People's	
1	Beijing YouAn Hospital, Capital Medical University	1
1	Beijing YouAn Hospital, Capital Medical University Puren Hospital Affiliated with Wuhan University of Science beijing youan houspital Wuhan PuRen Hospital Shanghai Guanghua integrated traditional Chinese	1

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1	Guangzhou No. 8 People's Hospital [Guangzhou, China], Wuhan Union Hospital, and Wuhan Hankou Hospital	1
	The First Affiliated Hospital of Guangzhou Medical University	
	Guangzhou Eighth People's Hospital	
	the Fourth People's Hospital of Nanning	1
	The fourth people's hospital of nanning, guangxi	
1	Dawu County People's Hospital	0
	Huangpi Chinese Medicine Hospital	
	The First Affiliated Hospital of Guangzhou Medical University	
	Wuhan University People's Hospital	
0	Not specified	1
1	Renmin Hospital of Wuhan University	1
	Renmin Hospital of Wuhan University	
	Zhongnan Hospital of Wuhan University	
1	Affiliated Tongji Hospital of Huazhong University of Science and Technology, Wuhan Institute for	
	People's Hospital of Wuhan University	
	Central Hospital of Chongqing Three Gorges	
	the First people's Hospital of Jingzhou	1

	Jingzhou First People's Hospital	
0	Not specified	1
	Guiqiang Wang Peking University First Hospital Ezhou Central Hospital Huoshenshan Hospital of	
1	CDC of Wuzhi Country	1
1	Wuzhi County Center for Disease Control and Prevention, Henan Province Wuzhi County Center for Disease Control and Prevention People's hospital, Wuzhi	1
	Shangqiu City Liangyuan District Center for Disease Control and Prevention	1
	the Shangqiu City Liangyuan District Center for Disease Control and Prevention	1
	Liangyuan district centers for disease control and prevention	
	Taizhou Vaccine Clinical Research Center	1
	Jiangsu Provincial Center for Disease Control and Prevention	
	Jiangsu Provincial Center for Disease Control and Prevention	1

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	Huaiyin District Center for Disease Control and Prevention, Huaian City	
	Huaiyin District Center for Disease Control and Prevention, Huaian City	
	Jiangsu Provincial Center for Disease Control and Prevention (JPCDC)	1
	Huaiyin District Center for Disease Control and Prevention, Huaian City	
	the First Affiliated Hospital of Bengbu Medical College	1
	The First Affiliated Hospital of Bengbu Medical College	
	the Fifth Medical Center of Chinese PLA General Hospital	1
	The Fifth Medical Center, General Hospital of PLA	
	Shanghai Public Health Clinical Center (SPHCC)	
	Shanghai Public Health Clinical Center	
	Guangzhou Eighth People's Hospital	1

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	Guangzhou Eighth People's Hospital	
0	Not specified	1
	Not specified	
	Shanghai Public Health Clinical Center	
	Shanghai Public Health Clinical Center	
1	Zhongnan Hospital of Wuhan University, Leishenshan (Thunder God Mountain) Hospital	1
	Zhongnan Hospital of Wuhan University	
0	Not specified	1
	Hubei province hospital of integrated Chinese & Western Medicine	
	Yichang first people's Hospital	
	The Second Affiliated Hospital of Wenzhou Medical University	1
	The Second Affiliated Hospital of Wenzhou Medical University	

0	Not specified	1
0	Not Specified	0
	General Hospital of Central Theater Command Maternal and Child Hospital of Hubei Province	
0	Not specified	1
	Hubei Provincial Center for Disease Control and Prevention	
	Jiangsu Provincial Center for Disease Control and Prevention (CDC) in Suining County Suining County Center for Disease Control and Prevention	
	Hebei Provincial Center for Disease Control and Prevention (CDC) in Renqiu (Hebei, China). Renqiu City Center for Disease Control and Prevention	
	Jiangsu Provincial Center for Disease Control and Prevention (CDC) Hebei Provincial CDC Suining County Center for Disease Control and Prevention Renqiu City Center for Disease Control and Prevention	

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1	the CDCs of Mile and Gejiu County of Yunnan Province	1
0	Not specified	1
	West China Second University Hospital, Sichuan University West China women's and children's Hospital	
0	Not specified	0
	Huashan Hospital affiliated to Fudan University	
1	The phase 1 trial was done at two hospitals in China: The Second Affiliated Hospital of Chongqing Medical The Second Affiliated Hospital of Chongqing Medical University Hunan Provincial Center for Disease Control and Prevention	1
0	Not Specified	1
	Jiangsu Provincial Center for Diseases Control and Prevention Jiangsu Provincial Center for Diseases Control and Prevention	
	The First Affiliated Hospital, Zhejiang University School of Medicine	1

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		The First Affiliated Hospital, Zhejiang University School of Medicine	
1		the Hongshan and Wuchang districts of Wuhan City, Hubei Province	1
0		Not Specified	1
		Hubei Provincial Hospital of TCM	
		Dongxihu FangCang hospital, Wuhan Keting, Hubei Province	1
		Wuhan Keting Square Cabin Hospital	
1		Leishenshan Hospital of Wuhan Tongji Hospital Wuhan Mental Health Center Huangshi Hospital of traditional Chinese Medicine Tongji Hospital of Huazhong University of science and	1
1		Wuhan JinYinTan Hospital	1
		Wuhan Jinyintan Hospital Renmin's Hospital of Wuhan University Tongji Hospital Affiliated to	
1		Peking University Third Hospital Peking University People's Hospital Tongji Hospital of Tongji Medical College, Huazhong University of Science and Technology	1

	Tongji Hospital of Tongji Medical College, Huazhong University of Science and Technology	
	Huangshi Hospital of Traditional Chinese Medicine	1
	Huangshi city hospital of TCM	
1	Wuhan Fangcang Shelter Hospital	
	First Affiliated Hospital of Anhui Medical University	
	Wuhan mobile cabin hospital	
	Binhu hospital of the first	
	the Wuhan Jinyintan Hospital	1
	Wuhan Jinyintan Hospital	
	the Endoscopy Center of West China Hospital, Sichuan University	1
	West China Hospital, Sichuan University	
	武汉市雷神山医院	1
	Leishenshan hospital	
	the First People's Hospital of Jiangxia District, Wuhan	

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	The first people's Hospital of Jiangxia District, Wuhan	
1	First Affiliated Hospital of Wenzhou Medical University	1
	The First Affiliated Hospital of Wenzhou Medical University Wenzhou city center hospital medical group	
	Jiangsu Province Hospital/Nanjing Medical University First Affiliated Hospital	1
	Jiangsu Province Hospital Hubei Province Hospital Intergrated Chinese & Western Medicine	
0	Not Specified	
	The First Affiliated Hospital of Guangzhou Medical University Xiangzhou District People's	
	Shulan (Hangzhou) hospital in Hangzhou	1
	Shulan (Hangzhou) Hospital	
0	Not Specified	
	Xiangyang Central Hospital Xiangyang Hospital of Traditional Chinese Medicine	
0	Not Specified	1

	Outpatient Department of Shanxi Aviation Industry Group Co. LTD Outpatient Department of Shanxi Aviation Industry Group Co. LTD	
0	Not Specified	
	The Second Hospital of Hebei Medical University	
0	Not Specified	1
	Gaozhou Municipal Center for Disease Control and Prevention	
	the Guangdong Provincial Center for Disease Control and Prevention Gaozhou Municipal Center for Disease Control and Prevention	1
1	Wuhan Pulmonary Hospital Ezhou Central Hospital The Third People's Hospital of Shenzhen The Second People's Hospital of Fuyang Ezhou Hospital of Traditional Chinese Medicine	1
0	Not Specified	
	Beijing SHIJITAN Hospital	

	Shanghai Public Health Clinical Center	1
	Shanghai Public Health Clinical Center	
	Shanghai Public Health Clinical Center	
	Zanhuang CDC	
	Zanhuang county Center for Disease Control and Prevention	
0	Not Specified	
	Taixing City center for Disease Control and Prevention	
	Lianshui Center for Disease Control and Prevention	
	Jiangsu Provincial Center for Disease Control and Prevention	
	the Second Hospital of Anhui Medical University of China	
	The Second Hospital of Anhui Medical University	
1	Huashan Hospital, National Medical Center for Infectious Diseases [NMCID], Shanghai, China	1

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Huashan Hospital affiliated to
Fudan University
Beijing Ditan Hospital Capital
Medical University

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Inclusion Criteria

Male and nonpregnant female patients 18 years of age or older were eligible if they had a diagnostic specimen that was positive on RT-PCR, had pneumonia confirmed by chest imaging, and had an oxygen saturation (Sao2) of 94% or less while they were breathing

1. Adult aged >=18years old;
2. Patients infected with new coronavirus confirmed by PCR
3. In the state of no oxygen at rest, the patient's SPO2<=94% or the oxygenation index is less than 300mmHg.

(1) 18–65 years of age; (2) diagnosed as mild to moderate COVID-19; and (3) willing to sign informed consent

(1) 18â€“65 years of age;
(2) Diagnosed as mild to moderate COVID-19;
(3) Be willing to sign informed consent;
(4) The clinical assessment and condition of the subject does not influence the completion of the study

adult patients (≥18 years),
positive test result for SARS-CoV-2 on a polymerase-chain-reaction (PCR) assay,
respiratory rate (RR) ≥30/min or SaO2 ≤ 93% or a PaO2/FiO2 ratio ≤ 300 mmHg (7),
and able to provide informed consent

1) Aged >= 18 years;
2) Meet the diagnostic criteria of severe pneumonia patients with new coronavirus;
3) The informed consent form.

Hospitalized COVID-19 patients with confirmed SARS-CoV-2 detection, clinically classified as moderate or severe, at an age over 18 years, and without comorbidity of severe heart, lung, or brain diseases, were eligible for enrolling into this study.

(1) aged 18 to 70 years;
(2) Light and heavy patients with new coronavirus infection confirmed by etiology.

(I) diagnosed with COVID-19;
(II) one or more symptoms among fever, cough, and fatigue at enrollment;
(III) the time from onset to diagnosis time was not more than 48 h; and
(IV) ≥18 years of age

(1) Comply with the diagnostic criteria of "Pneumonitis Diagnosis and Treatment Scheme for New Coronavirus Infection (Trial Version 5)";
(2) The classification was judged as common and severe type;
(3) Having one or more symptoms of fever, cough and fatigue when entering the group;
(4) The diagnosis time does not exceed 48 hours;

Eligible patients were males and non-pregnant females aged 18 years or older, diagnosed with moderate-to-severe COVID-19 pneumonia according to the Diagnosis and Treatment Protocol for Novel Coronavirus Pneumonia released by the National Health Commission of China.

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1. Age over 18 years old;
2. Real-time fluorescent RT-PCR for respiratory or blood specimens to detect novel coronavirus nucleic acid positive;
3. The sequence of virus genes in respiratory or blood samples was highly homologous with the known novel coronavirus;

Inclusion criteria were adults (aged >18 yr), with clinically-confirmed COVID-19 pneumonia and hypoxaemia (defined as the ratio of arterial oxygen tension [Pao 2] to inspiratory oxygen fraction [F io 2] <300 mm Hg), and requiring intubation in the ICU.

1. Eligible patients were adults ((aged 18-80 years);
2. Were identified as laboratory-confirmed 2019-nCoV infection;
3. Those who need intubation in ICU, and with no severe hypoxemia.

(1) signed informed consent;
(2) aged at least 18 years;
(3) COVID-19 diagnosis based on polymerase chain reaction (PCR) testing;
(4) positive PCR result within 72 hours prior to randomization;

1. The patient signed an informed consent form to participate in the study of convalescent plasma therapy;
2.The age of the patient is 18 or more than 18 years old;
3. COVID-19 patients diagnosed by PCR;
4. Nucleic acid positive within 72 hours before blood transfusion;

1) age 18 and over, regardless of gender;
2) respiratory or blood samples that were tested positive for SARS-CoV-2 nucleic acid by RT-PCR, or respiratory or blood samples that were tested highly homologous with the known SARS-CoV-2 by viral gene sequencing;

(1) aged >=18 years old;
(2) real-time fluorescence rt-pcr of respiratory or blood samples presents the positive nucleic acid of COVID-19, or the viral gene sequencing of respiratory or blood samples was highly homologous with the known COVID-19.
(3) patients diagnosed with novel coronavirus meet the diagnostic criteria of the latest

Inclusion criteria were age 18 years or older, ongoing SARS-CoV-2 infection confirmed in upper or lower respiratory tract specimens with real time reverse transcriptase polymerase chain reaction (RT-PCR), willingness to participate, and consent not to be enrolled in other clinical trials during the study period

1. Aged 18 years old and above;
2. Meet the novel coronavirus pneumonia (COVID-19) diagnostic criteria. The upper and lower respiratory tract RT-PCR confirmed that 2019-nCoV nucleic acid positive, chest CT imaging examination could be used in conjunction;
3. SaO2/SpO2=94% under indoor air, or PaO2/FiO2<300mgHg (for severe type patients);

We initially enrolled patients with COVID-19 (age 18-95 years) according to the guidance of National Health Commission of China (Table 1). If no improvement signs were observed under the standard treatments, the patient would be suggested to join this polit study.
(1) age range 18–95
(2) confirmed as infected with SARS-CoV-2 by qRT-PCR
(3) confirmed as having common, severe, or critically severe COVID-19 according to the guidelines of the National Health Commission of China

1. Male or female, 18-95 years old;
2. 2019 new coronavirus nucleic acid test positive;
3. Diagnosed as moderate to severe cases of new coronavirus pneumonia in 2019, which meets the following clinical manifestations:

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Patients 18 years or older who had positive test results with reverse-transcription polymerase chain reaction (RT-PCR) assay for severe acute respiratory syndrome coronavirus 2 (SARS-CoV-2) in oropharyngeal samples were eligible if they had pneumonia as confirmed by chest imaging and a PBL cell count of 800 per ul (to convert to ×109/l

1. Patients aged 15-80 years,diagnosed with mild, common, and severe novel coronavirus pneumonia in accordance with the diagnostic criteria for the diagnosis and treatment of novel coronavirus pneumonia issued by the National Health Commission (trial version 5);
2. White blood cell Count < 15.0 x 10^9/L and Lymphocyte Count < 0.8 x10^9/L.

had a diagnosis of moderate COVID-19 (combined with epidemic history, with fever, respiratory symptoms, nucleic acid test positive, radiological findings of pneumonia, but did not match severe or critical diagnosis); agreed to collect oropharyngeal swabs and venous blood

Patients aged >=18 years with novel coronavirus pneumonia who agreed to participate in this trial and signed the informed consent.

Not specified

Suspected cases of neo-coronary pneumonia (outside Hubei province), clinically diagnosed cases (in Hubei province), and light / general confirmed cases, severe/critical confirmed cases, aged 18-70 years, each center enrolls cases in a competitive manner.
The diagnostic criteria for the new coronavirus pneumonia conform to the diagnostic criteria issued by the National Health and Medical Commission for the diagnosis and treatment of

(1) Men or women, 18–70 years old; (2) meet the diagnostic criteria of a confirmed case according to national diagnostic and treatment program; (3) within 10 days of onset; (4) subjects who can take oral medication or receive oral medication; (5) non-pregnant women, and effective contraception within 7 days of the last drug use to ensure that no pregnancy is

(1) aged 18 -70 years with a diagnosis of COVID-19 conforming to the Chinese Guidelines;
(2) hospitalized for prolonged post-symptomatic viral shedding;
(3) able to orally take medication;
(4) non-pregnant women;

1. aged 18-70 years male or female;
2. 2019 new coronavirus nucleic acid test positive;
3. meet the national diagnosis criteria;
4. subjects onset within 8 days (severe cases can be delayed to 14 days);

The inclusion criteria those who were aged between 18-75 years (including threshold), regardless of gender; were under isolation observation while diagnosed with a mild or common type of COVID-19 and could cooperate to complete a corresponding psychological intervention; had mild to moderate depressive or anxiety symptoms as defined by the 17-

1. Age between 18 and 75 years old (including threshold), regardless of gender;
2. The patient under isolation observation is diagnosed with mild or common type of 2019-nCoV acute respiratory disease and can cooperate to complete corresponding psychological intervention;

1 Informed written consent from the patients themselves or their close relatives;
2 Age range from 18 to 80 years;
3 Diagnostic criteria for novel coronavirus pneumonia in the diagnosis and treatment guidelines for severe COVID-19 patient (version sixth) developed by the National Health

1. The patient signs the written informed consent;
2. Aged 18-80 years;
3. the novel coronavirus novel coronavirus infection diagnostic criteria for pneumonia, namely, the new coronavirus pneumonia diagnosis and treatment plan (trial version sixth). It conforms to any of the following: 1) in case of shortness of breath, RR >= 30 times / min; 2)

①Laboratory-confirmed cases according to Chinese guidelines of COVID-19;
②Male or female more than 18 years old;
③Increased interleukin-6

Clinically diagnosed with Corona Virus Disease 2019
Increased interleukin-6
Sign the informed consent
Subjects who can take medicine orally
Agree to collect clinical samples

Age range: healthy people aged 18-59 years old
General good health as established by medical history and physical examination;
Since December 2019, the participant has not gone to Hubei province, overseas, or to a village/community where there had been COVID-19 cases, has not contacted with

Age range: healthy people aged ≥18 years old
General good health as established by medical history and physical examination;
Since December 2019, the participant has not gone to Hubei province, overseas, or to a village/community where there had COVID-19 cases, have not contacted with confirmed or

1. Healthy subjects aged 6 years and above;
2. Request medical history and physical examination, and the researchers judged health status of subjects;
3. Since December 2019, subjects have not been to Hubei, overseas and

Eligible participants were healthy people aged 18–80 years, who were negative for serum-specific IgM/IgG antibodies against SARS-CoV-2, as measured by a commercial kit (Innovita, China) at the time of screening.
The enrolled participants were healthy individuals aged 3–17 years, who were negative for serum-specific IgM or IgG antibodies against SARS-CoV-2 N and S proteins (tested and verified with a commercial kit from Innovita, China) at the time of screening and vaccine inoculation on the same day

1. Healthy subjects aged 3 years and above;
2. By asking for a medical history and a physical examination, the researchers determined that the subjects were in good health;
3. From December 2019 to now, the subject has not been to hubei province, outside the country or in a village/community where there has been an outbreak. The subject has not

1. Male or female subjects of ≥18 years old and ≤55 years old with body mass index (BMI) ≥18 and ≤30 at the Screening Visit.
2. Individuals who are in good health condition at the time of entry into the trial as determined by medical history, physical examination (including vital signs, ECG) and

For adult group (age ≥18 and ≤55)

Male or female subjects of ≥18 years old and ≤55 years old with body mass index (BMI) ≥18 and ≤30 at the Screening Visit.
Individuals who are in good health condition at the time of entry into the trial as determined

Eligible participants were healthy adults aged 18 to 59 years who were seronegative for anti-SARS-CoV-2 immunoglobulin M and immunoglobulin G (IgG) and negative for SARS-CoV-2 nucleic acid as confirmed by pharyngeal swab reverse transcription polymerase chain reaction. To be included, participants were required to have an axillary temperature of

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1. Healthy permanent residents aged 18 years and above;
2. Subjects agree to sign the informed consent forms voluntarily;
3. Subjects are able to comply with the requirements of the clinical trial protocol;
4. Armpit temperature <=37.0 degree C;
5. Female subjects of childbearing age were not pregnant at the time of enrollment, were

1. Healthy permanent residents aged 18 years and above;
2. Subjects agree to sign the informed consent forms voluntarily;
3. Subjects are able to comply with the requirements of the clinical trial protocol;
4. Armpit temperature <= 37.0 degrees C;
5. Female subjects of childbearing age were not pregnant at the time of enrollment, were

Eligible participants were healthy individuals aged 18 through 59 years, and 60 years or more who were seronegative for SARS-CoV-2 IgM and IgG and negative for SARS-CoV-2 nucleic acid as confirmed by pharyngeal swab reverse transcription polymerase chain reaction (RT-PCR).

1. Healthy permanent residents aged 18 years and above;
2. Subjects agree to sign the informed consent forms voluntarily;
3. Subjects are able to comply with the requirements of the clinical trial protocol;
4. Armpit temperature <= 37.0 degrees C;

(1) positive for the COVID-19 by the real-time fluorescent reverse transcription polymerase chain reaction (RT-PCR) test for the detection of nucleic acid in respiratory specimens with throat swabs and sputum specimens,

(2) patients with COVID-19 who had mild symptoms in line with the diagnostic criteria of the

1. Patients with COVID-19 who had mild symptoms in line with the diagnostic criteria of the Chinese Management Guidelines for COVID-19 (version 6.0)
2. Good communication and understanding of Chinese
3. Aged 20-69 years

Participants were recruited from "suspected COVID-19 patients" in the Fifth Medical Center of Chinese PLA General Hospital who were newly tested positive for SARS-CoV-2.

Confirmed COVID-19 infection case in term of laboratory evidence;
80 years ≥ age ≥ 14years;
Within 72 hours after the onset of abnormalities shown by Chest radiology or several symptoms (fever and cough).

All the participants had laboratory-confirmed SARS-CoV-2 infection and were willing to participate the study, as evidenced by signing an informed consent.

The participants were diagnosed as COVID-19 pneumonia, according to the notice on printing and distributing the diagnosis and treatment plan of pneumonia with new coronavirus infection (trial version 4 or update version) made by National Health Commission of the People's Republic of China.

Written the informed consent

1) age between 18 and 80 years;
2) SARS-CoV-2 infection confirmed by real-time PCR (RT-PCR) from pharyngeal swab;
3) mild clinical status, defined as having mild clinical symptoms but no signs of pneumonia on imaging or moderate clinical status, defined as having fever, respiratory symptoms and

In sputum, throat swab, lower respiratory tract secretion, blood and other samples, the nucleic acid of the novel coronavirus was positive, or the sequencing of the virus gene was highly homologous with the known novel coronavirus
Age is between 18-80 years old, the weight is more than 30kg, and there is no limit for men and women

Eligible patients were men and non-pregnant women with COVID-19 who were aged at least 18 years and were RT-PCR positive for SARS-CoV-2, had pneumonia confirmed by chest imaging, had oxygen saturation of 94% or lower on room air or a ratio of arterial oxygen partial pressure to fractional inspired oxygen of 300 mm Hg or less, and were within

Age ≥18 years at time of signing Informed Consent Form
Laboratory (RT-PCR) confirmed COVID-19.
Lung involvement confirmed with chest imaging
Hospitalized with a SaO2/SPO2≤94% on room air or Pa02/Fi02 ratio <300mgHg
<12 days since illness onset

年龄≥18岁，根据“诊疗方案”确诊COVID-19并签署知情同意书

The participants were diagnosed as COVID-19 pneumonia, according to the notice on printing and distributing the diagnosis and treatment plan of pneumonia with new coronavirus infection (trial version 4 or update version) made by National Health Commission of the People's Republic of China;

Participants aged over 18;
(1) age ≥ 18 and < 80 years;
(2) RT-PCR positive for SARS-CoV-2;
(3) pneumonia confirmed by chest imaging and admission to the ICU;
(3) PaO2/FiO2(P/F) < 300 mmHg.
≥ 18 years old;
Diagnosed as serious or critical SARI (according to the 4th version of Diagnosis and Clinical management of 2019-nCoV infected pneumonia);
Being treated in the ICU.

Patients with COVID-19 pneumonia aged 18 years or older, admitted to the general wards for less than 72 h, and able to sign informed consent were eligible for this trial.

age more than 18 years old
accord with the clinical diagnosis and/ or etiological diagnosis diagnostic criteria of Novel coronavirus pneumonia (COVID-19).
admitted in the general wards
be able to sign informed consent

(1) age ≥ 18 years but ≤ 80 years;
(2) confirmed or clinically suspected mild or moderate coronavirus pneumonia (COVID-19), based on China’s Novel Coronavirus Pneumonia Diagnosis and Treatment Plan (Provisional 5th Edition); and
Aged between 18 and 80 years (including 18 and 80years, male or female).

One of them:
Laboratory (RT-PCR) and clinical symptoms confirmed case of novel coronavirus pneumonia (COVID-19)

(1) severe COVID-19 diagnosed after onset of disease;
(2) chest computed tomography (CT) imaging confirmed pneumonia combined with lung damage.

Not Specified

Male or female, aged at 18 years (including) -75 years old
Hospitalized
Laboratory confirmation of SARS-CoV-2 infection by reverse-transcription polymerase chain reaction (RT-PCR) from any diagnostic sampling source

Eligible participants were healthy adults aged 18 years or older, who were HIV-negative and previous SARS-CoV-2 infection-free, confirmed by commercial human immunodeficiency virus antibody detection kit (InTec products, Xiamen, China) and SARS-CoV-2 rapid test kit (Jinwofu, Beijing, China) using fingertip blood at screening. To be included, participants

Aged between 18 and 60 years.
Able to understand the content of informed consent and willing to sign the informed consent
Able and willing to complete all the secluded study process during the whole 6 months study follow-up period.
Negative in HIV diagnostic test

Participants were eligible if they were healthy and aged 18–59 years.

Healthy adults aged 18-59 years;
Proven legal identity;
Participants should be capable of understanding the informed consent form, and such form should be signed prior to enrolment ;

Eligible participants were healthy adults aged 60 years or older.

Healthy adults aged ≥60 years;
Be able to understand and sign the informed consent voluntarily;
Provide legal identification;

18-59
(1) Healthy subjects aged 18-59;
(2) Able to understand and sign the Informed Consent Form voluntarily;
(3) Provide legal proof of identity.

Healthy adults aged 18-59 years;
Proven legal identity;
Participants should be capable of understanding the informed consent form, and such form should be signed prior to enrolment ;

Healthy adults aged ≥60 years;
Be able to understand and sign the informed consent voluntarily;
Provide legal identification;

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Healthy volunteers aged 18 to 59 years were eligible for enrollment with written informed consent.

Healthy volunteers 18–59 years of age were eligible for enrollment after providing informed consent.

Phase Ia:

Healthy adults aged 18 to 59 years (including boundary values), both men and women.
Proven legal identity.
Participants should understand the contents of the informed consent form, the vaccine in

Not specified

Male and female subjects aged 18 to 45 years, inclusive;
The body weight no less than 50 kg for male subjects and no less than 45 kg for female subjects. Body mass index (BMI) = weight (kg)/square of height (m2), ranging from 18-28 kg/m2 (including the critical value);
Normal or abnormal but clinically insignificant physical examination, vital signs, laboratory

Phase I

1) According to the observation age of this clinical trial: 18 59 years old (including) adults;
2) The subjects voluntarily agreed to participate in the study, signed the informed consent form, and provided valid identification; they understood and complied with the requirements

Comply with the observation age of this clinical trial: 18-59 years old (both included) adults.
The subjects themselves voluntarily agreed to participate in the study, and signed an informed consent form, and can provide legal identification; understand and abide by the requirements of the trial protocol; can participate in a half-year follow-up.
Body temperature under armpit ≤37.3 °C.

Persons with full civil capacity aged 18-59 years (both included);
The subjects themselves voluntarily agreed to participate in the study, and signed an informed consent form, and can provide valid identification; understand and comply with the requirements of the trial protocol;
Body temperature under armpit ≤37.3°C;

Eligible participants in both trials were healthy population aged 18 years or older, who signed the informed consent, had negative HIV and SARS-CoV-2 (IgG and IgM) antibody testing results confirmed by rapid test kit using fingertip blood. Besides, a negative throat swab for SARS-CoV-2 nucleic acid, a clear chest CT image free from imaging features of

Male or female subjects of ≥ 18 years old with body mass index (BMI) ≥ 18.5 and ≤ 30 at the Screening Visit.
The subject can provide with informed consent and signs and dates a written informed consent form (ICF) prior to the initiation of any trial procedures.

Aged 18 years and above.
Able to understand the content of informed consent and willing to sign the informed consent.
Able and willing to complete all the secluded study process during the whole study follow-up period (about 7 months).

(1) Adults 18-85 years of age, either man or woman, who have signed the informed consent voluntarily;
(2) Confirmed as COVID-19: positive results of throat swab or blood samples by real-time RT-PCR assay for 2019-nCoV;

1. 18 to 75 years of age, male or female, willing to sign informed consent;
2. Tested positive for novel coronavirus infection after the onset of symptoms using a real time polymerase chain reaction (RT-PCR)-based diagnostic assay;
3. At least 2 viral nucleic acid tests are still positive under current antiviral therapy;

suspected and confirmed cases who met the above-mentioned suspected COVID-19 diagnostic criteria and had mild and common symptoms;

18–80 years of age, gender-neutral;

provision of informed consent.

(1) suspected and diagnosed cases of COVID-19 meeting the diagnostic criteria mentioned previously;

(2) 18–85 years old, regardless of sex;

(3) provided informed consent.

1. Suspected and confirmed cases of pneumonia in accordance with the above new signs of coronavirus infection and the standard of clinical common diagnosis;
2. Aged 18-65 years old;
3. Signing the general informed consent form.

Patients aged 18–75 years with mild COVID-19

(1) Comply with the diagnostic criteria for general type COVID-19 in the "Diagnosis and Treatment Program for COVID-19"(trial version 5th).

(2) Agree to participate in the study and the patient or legal guardian signs the informed consent form.

Hospitalized patients with COVID-19 were confirmed by pathogenic detection viral diagnostic techniques (nucleic acid test).

Meet any one of the criteria in the severe or critical category according to the national guideline of diagnosis and treatment

1. Hospitalized patients with novel coronavirus pneumonia confirmed by pathogenic detection
2. Meet any one of the criteria for severe type: 1) Respiratory distress: RP >= 30/min; 2) At rest, the oxygen saturation <= 93%; 3) Arterial partial pressure of oxygen (PaO2)/Fraction of

1. Patients diagnosed with critically ill COVID-19. It complies with the COVID-19 Critical and Critical Diagnostic Standards, namely “Pneumonitis Diagnosis and Treatment Scheme for Novel Coronavirus Infection (Trial Version 5);
3. Sign written informed consent. Patients who cannot sign informed consent must obtain

1. Severe patients diagnosed with novel coronavirus pneumonia (COVID-19);
2. Aged 35 years old to 74 years old;
3. Patients or authorized family members volunteered to participate in this study and signed informed consent. Note: Case definition: Severe NCP: Refer to the "New Coronavirus

in the study of moderate COVID-19:

male and nonpregnant female;

SARS-CoV-2 tested positive by a quantitative reverse-transcriptase polymerase-chain-reaction (qRT-PCR) assay or IgM / IgG Enzyme-Linked ImmunoSorbent Assay (ELISA);

1. Age 18 to 75 Yearsmale or female subjects
2. Diagnosed with COVID-19,according to "New Coronavirus Infected Pneumonia Diagnosis and Treatment Scheme (Pilot Sixth Edition);
3. Sever COVID-19, except shock and ICU care with organ failure;

1. Aged >=18 years at time of signing Informed Consent Form;
2. Patients with confirmed cases meeting the following criteria (1) Epidemiology history; (2) Clinical manifestations (epidemiological history and in accordance with any of the following 2 items; no clear epidemiology history, conforms to 3 items in clinical manifestations): Fever

(1) critical diagnostic criteria for severe COVID-19
(2) aged between 20 and 80 years
(3) stable condition and conscious and cooperative during the examination
(4) willing to participate in the trial and sign the informed consent form

(1) Meet NCP critical diagnosis criteria;
(2) Volunteer to participate in this study, cooperate with examination and treatment;
(3) The selected patients were basically stable, conscious and cooperative;
(4) No serious mental disorder;

(1) Patients ranging in age from 18 to 80 years (including boundary values) with no gender limitation
(2) Positive SARS-CoV-2 RNA detection from respiratory droplets by real-time polymerase chain reaction (RT-PCR)

(1) Age 18 to 80 years old (including threshold), sex unlimited, inpatient
(2) Respiratory specimen were positive for COVID-19 nucleic acid by RT-PCR;
(3) Clinical classification is mild or moderate type (with symptoms of fever, respiratory tract, imaging findings of pneumonia);

1) compliance with the diagnostic criteria for general type COVID-19 in the NHC-NATCM-China guidelines (version 6.0);
2) 18 ≤ aged ≤ 75 years;
3) Acceptance to participate in the trial by the patient, the legal guardian, or the person in

(1) Comply with the diagnostic criteria for general type COVID-19 in the "Diagnosis and Treatment Program for COVID-19"(trial version 6th).
(2) Aged >= 18 years.
(3) Agree to participate in the trial, and the patient, the legal guardian or the person in

(i) age of 18–80 years
(ii) outpatients undergoing gastroscopy
(iii) American Society of Anesthesiologists (ASA) physical status classification 1 and 2 patients at low risk of pulmonary aspiration

Patients undergoing painless gastroscopy in our hospital.

(1) 确诊为 COVID-19 普通型、重型，符合 《新型冠状病毒肺炎诊疗方案 (试行第六版) 》诊断标准，男女不限;
(2) 年龄≥18 岁且≤80 岁;
(3) 研究前详细了解试验性质、意义及可能的获益、不便、潜在危险，自愿签署知情同意书。

1. Real-time fluorescent RT-PCR detection of new coronavirus nucleic acid positive, or viral gene sequencing is highly homologous with known new coronavirus;
2. Confirmed as a novel coronavirus pneumonia common and severe patients, meets the diagnostic criteria of the "novel coronavirus pneumonia diagnosis and treatment plan (trial

All patients were confirmed as COVID-19 positive by either a reverse-transcriptase-polymerase-chain-reaction (RT-PCR) assay or clinical diagnosis according to the Guidelines for the Prevention, Diagnosis, and Treatment of Pneumonia Caused by COVID-19 (version 5) issued by the National Health Commission of the People's Republic of China.

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1. Novel coronavirus pneumonia is diagnosed met the fifth edition of the national health and Health Committee;
2. Respiratory rate in calm state >=30 times / min; OR Finger pulse oxygen saturation under rest and non oxygen absorption state <=93%; OR PO2/FiO2)<=300mmHg; OR

1. Age of 18-85 years, either male or female;
2. Diagnosed as positive for severe acute respiratory syndrome coronavirus 2 (SARS-CoV-2);
3. Symptomatic. Mild (mild clinical symptoms without signs of pneumonia on chest X-ray)

(1) Patients with Novel coronavirus pneumonia diagnosed according to <diagnosis and treatment scheme for pneumonia of new coronavirus infection (trial version 5)>;
(2) Inpatients aged >18 years old;
(3) Patients Know the informed consent, voluntary participate in this clinical trial and sign

Participants were aged 18–75 years, discharged from one of the participating hospitals after inpatient treatment for COVID-19, and had modified British Medical Research Council (mMRC) dyspnoea score of 2–3.

1. Discharged patients with COVID-19 pneumonia;
2. With dyspnea symptom (mMRC 2~3);
3. Aged 18~75 years old;
4. Have and can use smart phone (study subjects or family members);

hospitalised patients who met the following criteria:
(1)age ≥18 years;
(2) diagnosis of COVID-19 in accordance with “The Diagnosis and Treatment Scheme of COVID-19 (7th Trial Version)”;

1. Aged >= 18 years, male or female.
2. Patients who were diagnosed as COVID-19 according to the The Diagnosis and Treatment Scheme of COVID-19(The 7th Trial Edition).
3. The condition meets the discharge standards stipulated in the The Diagnosis and

Healthy adults aged 18-59 years;
informed consent;
good compliance;
axillary temperature <37.3 ℃

1. Healthy adults aged 18-59 who can provide the legal identification of the subjects, both male and female;
2. The subjects knew the content of the informed consent and the vaccination situation, voluntarily signed the informed consent, and had the ability to use the thermometer, scale

a. diagnosed with COVID-19 according to the Diagnosis and Clinical Management of 2019 novel coronavirus infected pneumonia (trial version 6 or trial version 7);
b. score of PCL-C≥50;
c. willing to participate in the study

1. According to the diagnosis and clinical management of New Coronavirus infection in 2019 (trial version 6 or trial version 7), COVID-19 was diagnosed;
2. Patients with pcl-c score >= 50;
3. Patients willing to participate in the study.

Eligible participants were airport ground staff and public security officers aged 18–59 years, without previous SARS-CoV-2 vaccination and infection, and negative for SARS-CoV-2 nucleic acid.

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1. Subjects aged 18-59 years;
2. The subjects who signed the informed consent form and participated voluntarily;
3. Subjects with good compliance;
4. Subjects with the interval of more than 14 days from the last vaccination;

1. Subjects aged 18-59 years;
2. The subjects who signed the informed consent form and participated voluntarily;
3. Subjects with good compliance;
4. Subjects with the interval of more than 14 days from the last vaccination;

(1) close contacts of COVID-19 cases, confirmed by a flow survey;
(2) at least 12 years of age, male or female;
(3) voluntarily signing a written informed consent form before the study began.

1. Confirmed as a close contact with the novel coronavirus pneumonia through flow investigation;
2. Over 12 years old (including 12 years old), no gender limit;
3. Voluntarily sign a written informed consent before the start of the study.

Eligible participants were younger adults aged between 18 and 59 years and older adults over 60 years without a history of either traveling in moderate-to-high risk areas or of contact with confirmed, asymptomatic, or suspected COVID-19 cases.

Subjects who comply with the following criteria will be enrolled:
1. Willing to participate in the study with informed consent;
2. Health male or female adults (i.e. aged 18 years and above);
3. Without a history of traveling or residence in domestic areas of high and moderate

Eligible participants were healthy adults aged over 18 YOA, with a body-mass index of 18–28 kg/m2, without a history of travelling in moderate to high-risk areas or a history of contact with confirmed, asymptomatic, or suspected COVID-19 patients.

Subjects who comply with the following criteria will be enrolled:
1. Willing to participate in the study with informed consent;
2. Health male or female adults (i.e. aged 18 years and above);
3. BMI within 18-28 kg/m2 (including both boundaries);

① Over 18 years of age, both male and female;
② After the first diagnosis and treatment of COVID-19, the SARS-CoV-2 RNA test of respiratory specimens such as sputum or nasopharyngeal swabs, has been negative for two consecutive times (sampling time interval of at least 24 h), in accordance with China COVID-

COVID-19 has been diagnosed, and the nucleic acid test of respiratory specimens such as sputum or nasopharyngeal swabs has been negative for two consecutive times after treatment (sampling time interval of at least 24 hours);
The nucleic acid test of specimens such as sputum, throat swabs, blood, feces and other

(i) age of ≥18 years;
(ii) body mass index (BMI) between 18.0 and 26.0 kg/m2,
(iii) healthy status as evaluated by previous medical history, physical examination, 12-lead electrocardiogram, chest CT scan, and laboratory tests;

Able to provide written informed consent
Males or females. Aged ≥ 18 years old
Body mass index (BMI) between 18.0 and 26.0 kg/m2
Normal or abnormal but non-clinical significant physical examination, vital signs, 12-ECG

Eligible participants were healthy adults aged 18–45 years as defined by the inclusion and exclusion criteria related to medical history, physical examination, and clinical laboratory tests, with no participation in any test drugs within the last three months.

Subjects are fully aware of the purpose, nature, methods and possible adverse reactions of the study, voluntarily serve as subjects, and sign the informed consent form before the start of any study procedures, and ensure that he/she will participate in any procedures in the study in person;

Patients diagnosed with mild or moderate COVID-19 (as per the Diagnosis and Treatment Protocol for Novel Coronavirus Pneumonia (Trial, 8th edition), China, or Clinical Management of COVID-19, WHO);

Has at least one COVID-19 symptom, e.g., fever, cough, shortness of breath, sore throat,

(1) Healthy subjects aged 3~17 years old;

(2) The subjects/guardians are able to understand and sign the informed consent voluntarily (both subjects and guardians should sign the informed consent for those aged 8~17 years old);

Healthy children and adolescents aged 3-17 years;

The subject and/or guardian can understand and voluntarily sign the informed consent form (double sign required for 8-17 years old);

Proven legal identity.

Aged 18 to 55 years old in MID cohort; aged 6-17 years old in MIN cohort; aged 56 years and above in OLD cohort;

Able to understand the content of informed consent and willing to sign the informed consent;

Able and willing to complete all the secluded study process during the whole 6 months study

Aged 18 to 55 years old in MID group; aged 6-17 years old in MIN group; aged 56 years and above in OLD group; aged in EBO group is not limited.

Able to understand the content of informed consent and willing to sign the informed consent

Able and willing to complete all the secluded study process during the whole 6 months study

1. Health subjects aged 18-59 years, who have been completed two-dose regimen of inactive SARS-CoV-2 vaccine in the past 3-6 months, or received one dose of inactive SARS-CoV-2 vaccine in the past 1-3 months.

2. The subject can provide with informed consent and sign informed consent form (ICF).

Health subjects aged 18-59 years, who have been completed two-dose regimen of inactive SARS-CoV-2 vaccine in the past 3-6 months, or received one dose of inactive SARS-CoV-2 vaccine in the past 1-3 months.

The subject can provide with informed consent and sign informed consent form (ICF).

1. Get adequate knowledge of the trial objective, and general knowledge of the pharmacological actions of the investigational drug, and potential adverse reactions; voluntarily sign the informed consent form in accordance with the principles of the Declaration

A signed informed consent form (ICF) from the patient or their legally authorized representative.

Healthy males or females, ages ≥18 to ≤45 years. Male subjects with weight ≥50 kg, or

1. Healthy adults, or adults with pre-existing but stable medical conditions (Participants didn't require significant change in therapy or hospitalization within 3 months before enrollment).

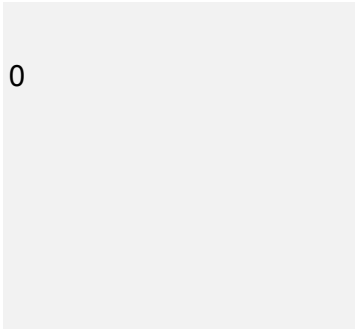
2. Participants who have received priming vaccination by two doses of inactivated whole-

Age ≥ 18 years old,
Through asking medical history and physical examination, the investigator judged that the health condition is good: the chronic disease is stable and there is no change within 3 months of medication;

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Deviation in Exclusion Criteria (1 if Yes, 0 if not specified in one or more of data sources)
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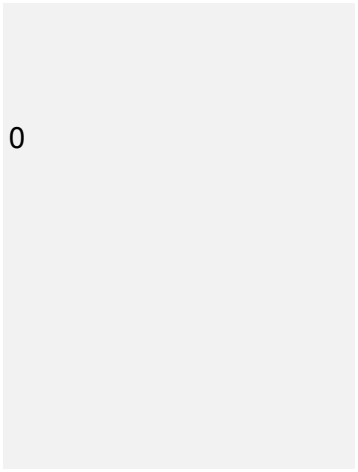
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Exclusion Criteria

Exclusion criteria included a physician decision that involvement in the trial was not in the patient's best interest, presence of any condition that would not allow the protocol to be followed safely, known allergy or hypersensitivity to lopinavir–ritonavir, known severe liver disease (e.g., cirrhosis,

1. Any situation that makes the programme cannot proceed safely;
2. Known allergy or hypersensitivity reaction to lopinavir / ritonavir;
3. Increase in alanine aminotransferase (ALT) / aspartate aminotransferase (AST) is more than 5 times the upper limit of normal;
4. Use of medications that are contraindicated with lopinavir / ritonavir and that cannot be replaced

(1) were pregnant or breastfeeding women; (2) had aspartate aminotransferase (AST) or alanine aminotransferase (ALT) >5× upper normal limit, creatinine clearance <50 ml/min (Lu and Dong, 2019); (3) were allergic or intolerant to therapeutic drugs; (4) were HIV-positive patients; (5) had

(1) Patients are diagnosed as a case of severe NCIP;
(2) Patients are pregnant or breastfeeding women;
(3) Patients have aspartate aminotransferase (AST) or alanine aminotransferase (ALT) > 5Ã—UNL or creatinine clearance <50 ml / min;
(4) Patients are allergic or intolerant to the proposed antiviral therapeutic drugs;

Patients were excluded if known life expectancy was 48 h or less, on home oxygen at baseline, pregnant or lactating, diagnosed with end-stage diseases, or having used immunosuppressants for 6 months or longer.

1) Severe basic diseases that affect survival, including: uncontrolled malignant tumors with multiple metastases that cannot be removed, hematological diseases, cachexia, active bleeding, severe malnutrition, HIV, etc.;
2) Obstructive pneumonia caused by lung tumors, severe pulmonary interstitial fibrosis, alveolar proteinosis, allergic reaction alveolitis;

Not specified

(1) any situation in which the program cannot proceed safely;
(2) allergic constitution;
(3) serious basic diseases affecting survival, including uncontrolled malignant tumors with multiple metastases that cannot be resected, blood diseases, malignant fluid, active bleeding, severe malnutrition, HIV, etc ; The main organ function is abnormal

(I) immunodeficiency, immunosuppressive drugs, or glucocorticoids within the past 3 months; (II) preparing for pregnancy, pregnant women, or lactating women; (III) allergies to more than two types of drugs or foods or allergies to ingredients of the trial drug; (IV) psychosis or those without self-knowledge ability; (V) estimated survival time less than 48 h from the start of screening; (VI) tracheal

(1) Immunodeficiency diseases, or those using immunosuppressive agents or glucocorticoids within the past 3 months;
(2) Pregnancies, pregnant women and lactating women;
(3) An allergic condition, such as a history of allergies to two or more drugs or foods, or a known allergy to the ingredients in this test);

We excluded patients if they presented with any condition that would not allow the protocol to be followed safely; had a history of allergy or hypersensitivity to interferons or any of the ingredients used in this trial; had a history of myocardial infarction and other serious cardiovascular diseases; were unable to receive nebulized compound; and/or voluntarily requested to withdraw from the trial.

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1. Pregnant or lactating women;
2. Patients who cannot be inhaled by atomization;
3. Patients who are intolerant or allergic to interferon preparations;
4. Other patients were judged unsuitable for inclusion in the trial by researchers.

Not specified

1. Intubation required immidiately in case of cardiac arrest or asphyxia;
2. Pregnancy, severe chronic respiratory, uncontrolled gastric reflux disease and any nasopharyngeal anatomical obstacle;
3. Those patients has intubated in ICU.

(1) pregnancy or lactation; (2) immunoglobulin allergy; (3) IgA deficiency; (4) preexisting comorbidity that could increase the risk of thrombosis; (5) life expectancy less than 24 hours; (6) disseminated intravascular coagulation; (7) severe septic shock; (8) PaO2/FIO2 of less than 100; (9) severe congestive heart failure; (10) detection of high titer of S protein–RBD-specific (receptor binding

1.The doctor believes that the patient is not suitable to participate in this trial, including those who may not cooperate, do not comply with the requirements of the procedure, or participate in this trial may put the patient in an unsafe situation.
2.Pregnant or lactation periods women;
3.Immunoglobulin allergy;

1) known or suspected allergy to the composition of azvudine tablets;
2) patients with malabsorption syndrome or any other condition that affects gastrointestinal absorption, the need for intravenous nutrition or an inability to take oral medication;
3) patients on anti-HIV treatment;

(1) known or suspected allergy to the components of Azvudine tablets;
(2) patients with malabsorption syndrome or any other condition that affects gastrointestinal absorption and requires intravenous nutrition or unable to take oral medication;
(3) patients who is currently receiving anti-hiv treatment;
(4) patients with one of the following conditions: respiratory failure and mechanical

Exclusion criteria were age below 18 years; severe conditions including malignancies, heart, liver, or kidney disease or poorly controlled metabolic diseases; unsuitability for oral administration; pregnancy or lactation; allergy to hydroxychloroquine; inability to cooperate with investigators due to cognitive impairments or poor mental status; severe hepatic impairment (for example, Child Pugh

1. Aged less than 18 years old;
2. Other serious medical diseases such as malignant tumor, heart, liver and kidney disease, uncontrollable metabolic disease, etc;
3. Not suitable for gastrointestinal administration;
4. Pregnant or lactating women;

Patients were ineligible if they had been diagnosed with any kind of cancers or the doctor declared the situation to belong to the critically severe condition. We excluded patients who were participating in other clinical trials or who have participated in other clinical trials within 3 months.

Patients were ineligible if they were pregnant, had been diagnosed as having cancer, or were experiencing critical respiratory failure.

1. Malignant disease in the past five years;
2. The clinical prediction is that there is no hope for survival, only cases of hospice care, or cases of deep coma that do not respond to supportive treatment measures within three hours of admission;
3. Subjects who are participating in other clinical trials or within 3 months;

Patients were excluded if they had a peripheral blood leukocyte cell count greater than 15000 per μL , any comorbidity (eg, chronic obstructive pulmonary disease, hypertension, diabetes, coronary heart disease, and/or malignancy), required invasive ventilation, developed shock or other organ failure that required admission to an intensive care unit, had an allergy to rhG-CSF, or were

1. Combining basic diseases such as hypertension, diabetes, and coronary heart disease;
2. Malignant tumor or with a history of malignant tumor;
3. pregnant or lactating women;
4. suffer from severe mental illness;
5. History of unstable angina pectoris or myocardial infarction within 6 months; coronary

Patients were excluded from consideration in the study if they had received systemic antiviral and immune regulation within 3 months; viral RNA nucleic acid test has turned negative after diagnosis; allergic to M. vaccae; had other severe disease such as severe heart disease, myocardial damage, significant vascular sclerosis, endocarditis, extremely weak, and severe anemia; pregnant women

1. Pregnant women;
2. Patients with a history of angina pectoris, myocardial infarction and other serious cardiovascular diseases;
3. The researchers judged that this study was not in the interests of patients.

Not specified

1. Patients with presence, coagulopathy or hypotension;
2. Pregnant and lactating women;
3. People with allergies or allergies to Xuebijing Injection and its components;
4. Severe basic diseases that affect survival, including: uncontrolled malignant tumors that have metastasized and cannot be removed, blood diseases, cachexia, active bleeding, severe

(1) cases of severe vomiting that are difficult to take pills or cases that have difficulties in absolving the medicine after oral administration;
(2) pregnant women and lactating women;
(3) subjects received specific antiviral medications such as lopinavir/lidonavir, ribavirin, monoclonal

(1) presence of any condition that would not allow the protocol to be followed, including known allergy to leflunomide, use of medications that are contraindicated with leflunomide and that could not be replaced or stopped during the trial period;
(2) pregnant or breast-feeding;

1. severe vomiting and difficulty in taking oral medication or absorbing in oral medication;
2. pregnant and nursing women;
3. subjects received lopinavir/ridonavir, ribavirin, monoclonal antibodies and other specific antiviral drugs in the week prior to admission;

The exclusion criteria were patients who were clearly diagnosed with a psychiatric disorder, including depression, bipolar disorder, etc, in the 6 months prior to their diagnosis of COVID-19; patients with psychotic symptoms; patients with HAMD17 score≥24 or HAMA score≥21; patients with a high risk of suicide, defined as having a history of attempts by suicide in the 6 months prior to

1. In the six months before the diagnosis of 2019-nCoV acute respiratory disease, the patient was clearly diagnosed as a psychiatric disorder, including depression, bipolar disorder and so on;
2. Patients with psychotic symptoms;
3. A history of suicide attempt in the 6 months before the study, or a score of, more than 3 on item 3

1 The patient had a history of allergy to XBJ injection. The allergic reactions to XBJ injection were rash and anaphylactic shock.
2 Pregnant women or breastfeeding mothers.
3 Severe underlying disease, malignant tumor, hematopathy or AIDS included

1. Those who are allergic to Xuebijing injection and its components;
2. Pregnant or lactating women;
3. Serious basic diseases affecting survival, including: malignant tumor, hematopathy, HIV, etc;
4. Expected death within 48 hours;
5. Patients who are participating in other clinical trials;

①Allergic to favipiravir or tocilizumab;
②Pregnant or lactating woman;
③ALT or AST> 5 times of upper limit of normal;

Cases of severe vomiting that make it difficult to take the drug orally

Allergic to Favipiravir or tocilizumab

Pregnant and lactating women

Subjects received specific antiviral drugs such as lopinavir / ritonavir, ribavirin, arbidol, chloroquine phosphate, hydroxychloroquine, and monoclonal antibodies within one week before admission

8.2 Exclusion criteria for the first dose

Confirmed, suspected, or asymptomatic COVID-19 cases;

Those with positive antibody tests of the COVID-19;

History of SARS virus infection (identified through self-report or on-site inquiry)

Exclusion criteria for the first dose

- Confirmed, suspected, or asymptomatic COVID-19 cases;
- Those with positive antibody tests of the COVID-19;
- History of SARS virus infection (identified through self-report or on-site inquiry)

1. Confirmed cases, suspected cases or asymptomatic cases with COVID-19 (refer to Information System of China Disease Prevention and Control);
2. COVID-19 antibody test positive;
3. Fever (armpit temperature > 37.0 degree C), dry cough, fatigue, nasal obstruction, runny nose,

Exclusion criteria were a history of travelling to Hubei Province (China), regions outside of China, or regions with reported COVID-19 cases from December, 2019; a history of infection with SARS-CoV; fever, cough, runny nose, sore throat, diarrhoea, dyspnoea, or tachypnoea in the 14 days before vaccination; abnormalities in laboratory tests; pregnancy or lactation; allergy to any ingredient

Exclusion criteria were a history of travelling to Hubei, regions outside of China, or regions within China with any reported COVID-19 cases since December, 2019;

a history of infection with SARS-CoV; fever (axillary temperature more than 37·3°C if aged older than 14 years, and axillary temperature of more than 37·5°C if aged 14 years or younger).

1. Confirmed cases, suspected cases or asymptomatic cases with COVID-19 (refer to Information System of China Disease Prevention and Control);
2. Positive in serum antibodies (IgG and IgM) screening of COVID-19;
3. Has a history of SARS virus infection (self-reported, site information);
4. Fever (armpit temperature > 37.0 degree C), dry cough, fatigue, nasal obstruction, runny nose

1. Have had any acute illness, as determined by the investigator, with or without fever, within 72h prior to the prime vaccination. An acute illness which is nearly resolved with only minor residual symptoms remaining is allowable if, in the opinion of the investigator, the residual symptoms will not compromise their well-being if they participate as trial subjects in the trial, or that could prevent, limit,

For adult group (age ≥18 and ≤55)

Have had any acute illness, as determined by the investigator, with or without fever, within 72 hours prior to the prime vaccination. An acute illness which is nearly resolved with only minor residual symptoms remaining is allowable if, in the opinion of the investigator, the residual symptoms will not

Individuals with confirmed cases, suspected cases, or asymptomatic cases of COVID-19 as referred to in the Information System of China Disease Prevention and Control were excluded. Those who had close contact with confirmed or suspected cases or had a history of travel to a foreign or domestic epidemic community within the 14 days before vaccination were also excluded. To be

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1. Within 14 days before vaccination, subjects have been abroad and to villages/communities experienced COVID-19 epidemics, and in contact with COVID-19 cases or suspected cases. Subjects are under isolation observation, or living in the villages/communities with COVID-19 cases or suspected cases;

2. Confirmed cases, suspected cases or asymptomatic cases with COVID-19 (refer to Information

1. Within 14 days before vaccination, subjects have been abroad and to villages/communities experienced COVID-19 epidemics, and in contact with COVID-19 cases or suspected cases. Subjects are under isolation observation, or living in the villages/communities with COVID-19 cases or suspected cases;

2. Confirmed cases, suspected cases or asymptomatic cases with COVID-19 (refer to Information

Individuals with confirmed cases, suspected cases or asymptomatic cases of COVID-19 as referred to the Information System of China Disease Prevention and Control were excluded. Those who had close contact with confirmed or suspected cases, or had travel history to a foreign or domestic epidemic community within 14 days before vaccination were also excluded. To be included,

1. Within 14 days before vaccination, subjects have been abroad and to villages/communities experienced COVID-19 epidemics, and in contact with COVID-19 cases or suspected cases. Subjects are under isolation observation, or living in the villages/communities with COVID-19 cases or suspected cases;

(1) previously diagnosed with depression and currently taking medication,
(2) had prior cognitive dysfunction and
(3) had experienced another major stressful event (e.g., divorce, bereavement) in the past year.

1. Previously diagnosed with depression and currently taking medication
2. Prior cognitive dysfunction
3. Experienced another major stressful event (e.g. divorce, bereavement) in the past year

(1) pregnant or lactating women;
(2) respiratory failure necessitating mechanical ventilation;
(3) liver failure (total bilirubin 10 mg/dL and/or severe coagulation disorders);
(4) renal function failure [urine 0.5 m•kg-1•h-1, creatinine (Cr) or blood urea nitrogen (BUN) 1.5×

Age < 14 years or > 80 years;

Pregnant or lactating female;

One of the following items occurred at the enrollment: (i) respiratory failure necessitating mechanical ventilation; (ii) liver failure: total bilirubin ≥ 10mg/dL and/or severe coagulation disorders; (iii) renal function failure; although adequate circulating blood and cardiac output, urine < 0.5ml/kg·h, Cr or

hypersensitivity to darunavir, cobicistat, or any excipients;

patients with severe liver injury (Child-Pugh Class C);

patients receiving concomitant medications that are highly dependent on cytochrome P450 3A clearance, and for which the elevated plasma concentrations are associated with serious or life-

Hypersensitivity to darunavir, cobicistat, or any excipients

Patients with severe liver injury (Child-Pugh Class C)

Concomitant medications that are highly dependent on CYP3A clearance, and the elevated plasma concentrations are associated with serious or life-threatening events.

Subjects were considered to be unable to complete the study, or not suitable for the study by

1) known or suspected to be allergic to LPV/r or arbidol;

2) having severe nausea, vomiting, diarrhea or other complaints affecting oral intake or absorption in the digestive tract;

3) taking other drugs that may interact with LPV/r or arbidol;

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Have a clear history of lopinavir or ritonavir or arbidol allergy
Severe nausea, vomiting, diarrhea and other clinical manifestations affect the oral or absorption of the drugs
At the same time, take drugs that may interact with lopinavir or ritonavir or arbidol
Patients with serious underlying diseases, including but not limited to heart disease (including

Exclusion criteria included pregnancy or breast feeding; hepatic cirrhosis; alanine aminotransferase or aspartate aminotransferase more than five times the upper limit of normal; known severe renal impairment (estimated glomerular filtration rate <30 mL/min per 1·73 m2) or receipt of continuous renal replacement therapy, haemodialysis, or peritoneal dialysis; possibility of transfer to a non-

Physician makes a decision that trial involvement is not in patients' best interest, or any condition that does not allow the protocol to be followed safely.
Severe liver disease (e.g. Child Pugh score ≥ C, AST>5 times upper limit)
Pregnant or breastfeeding, or positive pregnancy test in a predose examination
Patients with known severe renal impairment (estimated glomerular filtration rate <30 mL /min/1.73

1 对氯喹、羟氯喹过敏的患者；2 妊娠期妇女；3 合并心、肺、肾、脑、血液等重要器官严重疾病伴功能不全的患者；4 视网膜疾病、听力减退或听力丧失的患者；5 严重神经或精神疾病的患者；6 研究者认为无法按要求完成研究或不适宜参与研究者

Hypersensitivity to chloroquine or hydroxychloroquine;
Women during pregnancy;
Severe heart, lung, kidney, brain, blood diseases or other important systemic diseases;
Participants with retinal disease, hearing loss;
Participants with severe neurological and mental illness;

Exclusion criteria were allergy to vitamin C, pregnancy or breastfeeding, expected survival duration < 24 h, and previous history of glucose-6-phosphate dehydrogenase deficiency or end-stage pulmonary disease. Patients who were already enrolled in other clinical trials were excluded as well.

Allergic to vitamin C;
Dyspnea due to cardiogenic pulmonary edema;
Pregnant or breastfeeding;
Expected life is less than 24 hours;
There is a state of tracheotomy or home oxygen therapy in the past;

Exclusion criteria included severe immunosuppression (human immunodeficiency virus infection and long-term use of immunosuppressive agents), pregnant or breastfeeding women, corticosteroid needed for other diseases, refractory hypertension, epilepsy or delirium, glaucoma, active gastrointestinal bleeding within 3 months, refractory hypokalemia, secondary bacterial or fungal

severe immunosuppression (HIV infection, long-term use of immunosuppressive agents
pregnant or lactation period women
glucocorticoids are needed for other diseases
unwilling or unable to participate or complete the study
participate in other study

(1) alanine aminotransferase ≥ 5 × the upper limit of normal (ULN), total bilirubin level ≥ 3× ULN, or creatinine ≥ 1.5 × ULN;
(2) severe liver disease;
(3) severe or critical cases;

ALT≥5 times of ULN, level of total bilirubin≥3 times of ULN, or Cr≥1.5 times of ULN.
Patients with serious severe liver disease.
Excluded who diagnosed as pneumonia patients with novel coronavirus infection of severe type and critical type. 【Diagnostic criteria reference the official guideline "Pneumonia Diagnosis and Treatment Scheme for Novel Coronavirus Infection (Trial Version 5) "】

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The exclusion criteria included patients with shock or COVID-19 combined with any one of other organ failures, those who received invasive ventilation, or patients with any malignant tumor, pregnancy or breastfeeding, or co-infection of other pathogens.

Not Specified

Pregnancy, lactation and those who are not pregnant but do not take effective contraceptives measures;
Patients with malignant tumor, other serious systemic diseases and psychosis;
Patients who are participating in other clinical trials;

-Family history of seizure, epilepsy, brain or mental disease
- Participant that has an allergic history to any ingredient of vaccines
- Woman who is pregnant, breast-feeding or positive in pregnancy test on day of enrollment, or is planning to be pregnant during the next 6 months

Family history of seizure, epilepsy, brain or mental disease
Subject allergic to any component of the investigational vaccine, or a more severe allergic reaction and history of allergies in the past.
Woman who is pregnant, breast-feeding or positive in β-HCG (human chorionic gonadotropin) pregnancy test (urine) on day of enrollment, or become pregnant during the next 6 months

(1) Travel / residence history of Wuhan city and surrounding areas or other communities with case reports within 14 days prior to the entry;
(2) Contact with SARS-CoV-2 infected persons (positive for nucleic acid detection) within 14 days prior to the netry;

Travel history / residence history of Wuhan city and surrounding areas, or other communities with case reports within 14 days;
History of contact with a SARS-CoV-2 infection (positive in nucleic acid test) within 14 days;
Have contacted patients with fever or respiratory symptoms from Wuhan and surrounding areas, or from communities with case reports within 14 days;

(1) Travel / residence history of Wuhan city and surrounding areas or other communities with casereports within 14 days prior to the entry;
(2) Contact with SARS-CoV-2 infected persons (positive for nucleic acid detection) within 14 days prior to the netry;

Travel / residence history of Wuhan city and surrounding areas or other communities with case reports within 14 days prior to the enrolment;
Contact with SARS-CoV-2 infected persons (positive for nucleic acid detection) within 14 days prior to the enrolment;
Contact patients with fever or respiratory symptoms from Wuhan city and surrounding areas, or from

18-59
(1) Having traveled to or lived in Wuhan or surrounding areas or communities where confirmed cases have been reported within the previous 14 days;
(2) Having contact with patients who were infected with COVID-19 (who have tested positive for
Exclusion Criteria:

Travel history / residence history of Wuhan city and surrounding areas, or other communities with case reports within 14 days;
Exclusion Criteria:

Travel / residence history of Wuhan city and surrounding areas or other communities with case reports within 14 days prior to the enrolment;

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Not specified

Not specified

Phase Ia:

Contraindications for vaccination.
History of allergy to vaccines or drugs.
Immunization with any vaccine within 1 month

Not specified

Excluded for novel coronavirus (SARS-CoV-2) infection

Having one of the following evidence on SARS-CoV-2 infection:

SARS-CoV-2 determined by reverse transcription-polymerase chain reaction (RT-PCR) and/or next

Phase I

1) The vital signs, physical examination and laboratory test indexes specified in the protocol were abnormal, which were judged by clinicians to be of clinical significance;
The vital signs, physical examination and laboratory test indicators of the population specified in the plan are abnormal and have clinical significance as determined by the clinician;
Have a history of severe allergies to any component of the research vaccine, including aluminum preparations, such as: anaphylactic shock, allergic laryngeal edema, allergic purpura, thrombocytopenic purpura, local allergic necrosis (Arthus reaction); or any previous History of
The vital signs and physical examination results of the population specified in the plan have clinical significance as determined by the clinician;
A history of severe allergies to any component of the test vaccine, including aluminum preparations, such as: anaphylactic shock, allergic laryngeal edema, allergic purpura, thrombocytopenic purpura, local allergic necrosis (Arthus reaction), dyspnea, vascular neuropathy Edema, etc.; or any previous

Pregnant and breastfeeding women, people with allergic history, convulsion, epilepsy, encephalopathy, mental illness, acute infection, severe and uncontrollable cardiovascular disease or other chronic illnesses were excluded.
Exclusion criteria of prime dose:

Subjects with a medical or family history of convulsions, epilepsy, encephalopathy, and psychosis.
Allergic to any ingredient in the study vaccine, or used to have a serious vaccine allergic reaction.
First dose exclusion criteria:

Positive serum immunoglobulin M (IgM) and IgG to the SARS-CoV-2.
A Known History of HIV infection

(1) Allergic constitution, known to be allergic to baloxavir marboxil or favipiravir or pharmaceutical excipients;
(2) Weight < 40 kg;
(3) Critical illness meeting one of the following conditions: respiratory failure and mechanical

1. Allergic constitution, known to be allergic to balosavir or farpiravir or pharmaceutical excipients;
2. Body weight <40 kg;
3. Considered as severe disease: has an ongoing respiratory deficiency and subjected to invasive mechanical ventilation; or presence of shock; or admitted to the ICU due to complications;

Patients excluded from the study on the basis of the following criteria:
clear evidence of bacterial infection;
severe primary diseases of the heart, kidney, lung, endocrine, blood, metabolism, and gastrointestinal tract that may affect the patients' participation in the trial or the outcome of the study,

(1) clear evidence of bacterial infection;
(2) severe primary diseases, such as heart, kidney, lung, endocrine, blood, metabolism, or gastrointestinal tract diseases, which may affect the patient's participation in the trial or affect the outcome of the study;

1. Clear evidences?of bacterial infection;
2. Serious primary diseases judged by researchers,such as heart, kidney, lung, endocrine, blood, metabolism and gastrointestinal tract,?may affect the patients' participation in the trial or the outcome of the study;

could not guarantee that medication would be taken during the trial observation period;
more than 7 days after diagnosis;
severe primary respiratory diseases;
pregnant or planning pregnancy;

(1) Patients who can not guarantee compliance of using TCM during the treatment period, or patients who are difficult to take medicine by oral or nasal route.
(2) Patients who have been more than 7 days since definite diagnosis.
(3) Patients with severe primary respiratory disease or other pathogenic microbial pneumonia that

1 Pregnant or lactating women
2 Allergy to herbs of Shenhuang Granule
3 History of a severe primary disease, including unresectable tumors, blood diseases (such as acute leukemia and purpura hemorrhagica) , HIV, severe liver dysfunction (serum bilirubin level > 205.2

1. Pregancy or lactating women;
2. People with allergies or allergies to components of Truncated Torsion formula;
3. Patients with serious basic diseases such as malignant tumor, cirrhosis, chronic renal failure (uremic stage), hematological disease, HIV, etc.;

(1) Patients who are participating in other clinical trials;
(2) Pregnant or breastfeeding women;
(3) There are other life-threatening diseases such as cancer;
(4) Expected survival time <24 h;

1. Involving in other drug clinical trial;
2. Pregnant or breastfeeding;
3. Presence of other serious diseases that may be life threatening, such as cancer;
4. Expected life is less than 24 hours;

a known allergy to 4-aminoquinolines drugs or analogs;
any comorbidities including hematological diseases, mental illnesses, retinopathy, maculopathy, severe liver diseases, hypokalemia, hypomagnesemia, pancreatitis, severe renal impairment, and glucose-6-phosphate dehydrogenase deficiency;

1 Patients with identified allergies to 4-aminoquinoline and lopinavir/litonavir;
2 Patients with server diseases of the blood system;
3 Patients with identified severe liver disease, underlying cirrhosis or alanine aminotransferase (ALT)/aspartate aminotransferase (AST) elevation more than 3 times the normal upper limit;

1. Patients with identified allergies to 4-aminoquinoline and lopinavir/litonavir;
2. Patients with server diseases of the blood system;
3. Patients with identified severe liver disease, underlying cirrhosis or alanine aminotransferase (ALT)/aspartate aminotransferase (AST) elevation more than 3 times the normal upper limit;

(1) complications due to other underlying severe diseases, such as chronic obstructive pulmonary disease, obstructive pulmonary disease, coronary heart disease, and hypertension,
(2) serious mental illness,
(3) cognitive dysfunction preventing an understanding of the trial process and rehabilitation content,

1. Patients with other serious diseases such as lung diseases, cardiovascular and cerebrovascular diseases, hematopoietic diseases, autoimmune diseases, digestive system, or mental illness;
2. Pregnant or lactating women;
3. Participate in other forms of exercise during the trial

(1) Known or suspected to be allergic to the components of the test drug.
(2) Patients with confirmed hepatitis B virus, hepatitis C virus, human immunodeficiency virus (HIV), or other viral infections.
(3) Patients classified as severe or critical types with complications such as respiratory failure and

(1) Patients with known or suspected to be allergic to the components of the test drug;
(2) Patients with known hepatitis B, C, AIDS and other viral infections;
(3) Patients with severe or severe clinical classification who have experienced complications such as respiratory failure and acute respiratory distress syndrome (ARDS);

1) critical patients;
2) patients who could not guarantee compliance of using Q-14 during the treatment period, or patients who had difficulty taking drugs via the oral or nasal route;
3) patients with severe primary respiratory disease or other pathogenic microbial pneumonia with

(1) Critical patients.
(2) Patients who can not guarantee compliance of using TCM during the treatment period, or patients who are difficult to take medicine by oral or nasal route.
(3) Patients with severe primary respiratory disease or other pathogenic microbial pneumonia that

(i) pregnant women,
(ii) patients undergoing colonoscopy at the same time
(iii) suspected or confirmed patients with COVID-19.

1. Patients undergoing general gastroscopy;
2. With trachea dysfunction;
3. pregnant women, children, age more than 80 years;
4. Patients with difficulty in intubation.

(1) 轻型、危重型 COVID-19;
(2)任何其他影响临床试验评估的呼吸道疾病;
(3) 合并哮喘，需每日治疗;
(4) 胸部 CT 证实存在严重的肺间质病变、支气管扩张等肺部疾病;

1. Patients with mild and critical novel coronavirus pneumonia;
2. Acute respiratory disease caused by non- novel coronavirus pneumonia;
3. Patients with diabetes and hypertension accompanied by severe primary immunodeficiency disease acquired immunodeficiency syndrome congenital respiratory malformation congenital heart

Main exclusion criteria included age <18 years old, pregnancy, history of arrhythmia, chronic respiratory failure caused by other diseases such as heart failure, thoracic deformity, structural lung disease, hemodynamic instability, severe immunodeficiency, recent use of immunosuppressants, allergies, estimated survival time <3 days, withdrawal from research or return visits.

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1. Aged < 7-year-old;
2. previous arrhythmia;
3. previous chronic respiratory failure caused by other diseases, such as heart failure, thoracic deformity, structural lung disease, etc.;

1. < 18 or > 85 years old;
2. Pregnancy and lactation;
3. Serious heart, liver, kidney and hematopoietic system diseases, abnormal liver or kidney function;
4. Suffering from pneumonia caused by other known viruses;

(1) Death is expected within 48 hours;
(2) Basic diseases such as primary immunodeficiency disease, acquired immunodeficiency syndrome, congenital respiratory malformation , congenital heart disease, gastroesophageal reflux disease, congenital pulmonary airway malformation and more with the exact evidence of bacterial

patients with mMRC dyspnoea of 4–5 were excluded for reasons of safety.
Other exclusion criteria were: resting heart rate over 100 bpm, uncontrolled hypertension, uncontrolled chronic disease (eg, diabetes with random blood glucose >16.7 mmol/L, haemoglobin A1C >7.0%), cerebrovascular disease within 6 months, intra-articular drug injection or surgical

1. Dyspnea scale mMRC 0~1 or 4;
2. Resting heart rate>100bpm;
3. Adjusting dosage of drugs which affect cardiopulmonary function or heart rate (such as: trimetazidine, salbutamol, beta-blocker);

(1) known or suspected allergy to the components of BFHX;
(2) acute infections caused by other viruses or bacteria;
(3) abnormal liver and kidney function tests (alanine aminotransferase [ALT], aspartate aminotransferase [AST], and serum creatinine [SCr] ≥1.5 times the upper limit of normal);

1. Known or suspected allergy to the components of BFHX.
2. Acute infections, along with other viruses or bacteria.
3. Abnormal liver and kidney function tests (ALT, AST, SCr >= 1.5 times the upper limit of normal).
4. Participating in other drug clinical trials.

Pulse <50 or >100 beats/min;
SBP≥140 mmHg or DBP≥90 mmHg; BMI <18 or >30 kg/m2;
clinical screening laboratory evaluations are 1.2 times the normal reference range
Pregnancy or lactation or be menstruating or egg or sperm donation

Subjects meeting any of the following exclusion criteria were not included in the study
1. The subjects who did not meet the health standards in the comprehensive physical examination mainly included: (1) Patients with abnormal vital signs and clinical significance (awake pulse < 50 beats / min or > 100 beats / min, systolic blood pressure >= 140 mmHg or diastolic blood pressure

a. history of mental illness;
b. inability to use the internet.

1. Patients with mental history;
2. Patients who do not use the Internet.

(1) history or family history of allergy, convulsion, epilepsy, encephalopathy or psychosis;
(2) any intolerance or allergy to any component of the vaccine;
(3) known or suspected diseases including severe respiratory disease, severe cardiovascular disease, severe liver or kidney disease, medically uncontrollable hypertension (systolic blood

1. Patients with history of allergy, convulsion, epilepsy, encephalopathy and psychosis or family history;

2. Those who are allergic to any component of the vaccine, and those who have had any history of severe allergy to the vaccine (such as acute allergic reaction, urticaria, skin eczema, dyspnea,

1. Patients with history of allergy, convulsion, epilepsy, encephalopathy and psychosis or family history;

2. Those who are allergic to any component of the vaccine, and those who have had any history of severe allergy to the vaccine (such as acute allergic reaction, urticaria, skin eczema, dyspnea,

(1) COVID-19 infection confirmed by etiological tests and clinical manifestations or signs;

(2) an allergic constitution, such as an allergy history to two or more drugs or food or a known allergy to the ingredients of this drug;

(3) pregnancy or lactation.

1. Confirmed as a confirmed case of new coronavirus pneumonia after pathogenic testing and clinical manifestations or signs;

2. Allergies, such as those with a history of allergies to two or more drugs or foods, or those who are known to be allergic to the ingredients of this drug;

a history of COVID-19 or positive COVID19 screening test (reverse transcription polymerase chain reaction or enzyme-linked immunosorbent assay [ELISA]) results;

a history of SARS, autoimmune disease, confirmed or suspected immunosuppressive or immunodeficiency disorder, known allergic reactions to vaccines or vaccine components, or any

Subjects who meet any of the following criteria will be excluded:

1. Confirmed or asymptomatic COVID-19 cases;

2. Positive SARS-CoV-2 nucleic acid test;

3. Positive SARS-CoV-2 IgG or IgM test (reference to nucleic acid test and chest CT if necessary);

participants with a history of COVID-19 or those who tested positive at screening (by RT–PCR or ELISA);

with a history of SARS, autoimmune diseases, severe allergy reactions to vaccines, congenital or acquired angioneurotic oedema;

Subjects who meet any of the following criteria will be excluded:

1. Confirmed or asymptomatic COVID-19 cases;

2. Positive SARS-CoV-2 nucleic acid test;

3. Positive SARS-CoV-2 IgG or IgM test (reference to nucleic acid test and chest CT if necessary);

① Allergic to favipiravir;

② Pregnant or lactating women;

③The researchers considered the patient was not suitable to participate in this clinical trial.

Those allergic to fapilavir;

Pregnant or lactating women;

Unstable liver, kidney, and heart diseases;

History of mental disorders, substance abuse or dependence;

(i) allergy to humanized monoclonal antibodies and any ingredient of SCTA01;

(ii) suspected or verified SARS-CoV-2 infection;

(iii) a history of severe allergies;

(iv) infection or fever within 14 days before enrollment;

Having a history of severe allergy, such as severe allergic reactions, urticaria, angioedema;

Having one of the following evidence on SARS-CoV-2 infection(previous tests were accepted) :

SARS-CoV-2 positive: reverse transcription-polymerase chain reaction (RT-PCR) and/or next

Not Specified

Health status: clinically significant histories of heart, liver, kidney, digestive tract, nervous system, respiratory system, blood and lymphatic system, immune system, mental, metabolic, and bone abnormalities.

Subjects who have a history of allergies to biological agents or any drug components; those who

A subject who was diagnosed with severe or critical COVID-19 (as per the Diagnosis and Treatment Protocol for Novel Coronavirus Pneumonia (Trial, 8th edition), China, or Clinical Management of COVID-19, WHO);

Abnormal important organ function indicators, which meet the following conditions:

(1) Travel / residence history of communities with COVID-19 case reports within 14 days prior to the entry;

(2) Contact with SARS-CoV-2 infected persons (positive for nucleic acid detection) within 14 days prior to the entry;

Travel history / residence history of communities with case reports within 14 days;

History of contact with a SARS-CoV-2 infection (positive in nucleic acid test) within 14 days;

Have contacted patients with fever or respiratory symptoms from communities with case reports within 14 days;

Laboratory abnormalities of clinical significance, including white blood cell count, lymphocyte count, neutrophils, platelets, hemoglobin, alanine aminotransferase (ALT), aspartate aminotransferase (AST), total bilirubin, fasting blood glucose, and creatinine. Only apply to the sentinel participants in each cohort (the first 30 subjects);

Exclusion Criteria:

Family history of seizure, epilepsy, brain or mental disease

Subject allergic to any component of the investigational vaccine, or a more severe allergic reaction

1. have the medical history or family history of convulsion, epilepsy, encephalopathy and psychosis.

2. be allergic to any component of the research vaccines, or used to have a history of hypersensitivity or serious reactions to vaccination.

3. women with positive urine pregnancy test, pregnant or breast-feeding, or have a pregnancy plan

have the medical history or family history of convulsion, epilepsy, encephalopathy and psychosis.

be allergic to any component of the research vaccines, or used to have a history of hypersensitivity or serious reactions to vaccination.

women with positive urine pregnancy test, pregnant or breast-feeding, or have a pregnancy plan

1. Subjects receiving a surgery within 8 weeks prior to screening, or who are planned to receive a surgery prior to completion of the trial, which may cause unacceptable risks to the subjects as confirmed by investigators;

2. Use of the following drugs or therapies prior to administration of the investigational drug:

Those who have had surgery within 8 weeks prior to screening or who have surgery planned during the trial, and the investigator believes that such surgery may bring potential risks to the subjects.

Prior to administration of the investigational drug, use of the following drugs or therapies: i. any prescription drug within 28 days. ii. any over-the-counter drug, including health care products within

1. SARS-CoV-2 infection confirmed by positive reverse transcription-polymerase-chain-reaction (RT-PCR) assay.

2. A history of infection with SARS-CoV-2, or a history of contacting with cases of confirmed or suspected SARS-CoV-2 infection.

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RT-qPCR detects active SARS-Cov-2 infection;
a history of COVID-19, SARS, MERS infection (self-report, on-site inquiry);
a high-risk factor for COVID-19 infection:
a history of close contact with people infected with the new coronavirus, a history of living in high-

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Deviation in Masking (1 if Yes, 0 if not specified in one or more of data sources)	Masking	Deviation in Primary outcome (1 if Yes, 0 if not specified in one or more of data sources)
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Primary Outcome

time to clinical improvement, defined as the time from randomization to an improvement of two points (from the status at randomization) on a seven-category ordinal scale or live discharge from the hospital, whichever came first.

Clinical improvement time of 28 days after randomization

The 7-point scale

7 points: death

6 points: admission to ECMO and / or mechanical ventilation

5 points: Hospitalized for non-invasive ventilation and / or high-flow oxygen

the difference in the interval from baseline (initiation of antiviral treatment) to SARS-CoV-2 nucleic acid negativity by nasopharyngeal swab among the three antiviral treatment groups, with each of these two tests at least 24 h apart.

The time to 2019-nCoV RNA negativity in patients

the change in the disease severity category of COVID-19 after treatment. The severity of COVID-19 was assessed based on the Six-Point Clinical Status Scale for COVID-19 (COVID-19 severity scale) (Box 1). The Six-Point Clinical Status Scale for COVID-19 was defined according to NHC-NATCM-China

Critically ill patients (%)

the SARS-CoV-2 clearance rates in COVID-19 patients assessed on day six of antiviral treatment.

novel coronavirus nucleic acid clearance rate

the rate of achieving clinical symptom recovery on day 14 of treatment. The standard of clinical symptom recovery was the disappearance of the main symptoms, such as fever, cough, and fatigue.

Clinical symptom recovery rate

the time to clinical improvement, defined as the time from enrolment to an improvement of two points on a seven-category ordinal scale or live discharge from the hospital, whichever came first.

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disease remission
time to clinical improvement
overall rates of clinical improvement assessed at days 7, 14, and 28
time to chest CT improvement
time to virus nucleic acid negative-conversion

the total time of intubation, defined as the sum of the time spent from the beginning of bronchoscopy until proper tracheal tube placement was confirmed.

the lowest SpO2 during intubation

time to clinical improvement within a 28-day period. Clinical improvement was defined as patient discharge or a reduction of 2 points on a 6-point disease severity scale

the number of days between randomised grouping and clinical improvement

the rate of nucleic acid negativity conversion of SARS-CoV-2 and the negativity conversion time.

time and rate of temperature return to normal
time and rate of improvement of respiratory symptoms and signs (lung rhones, cough, sputum, sore throat, etc.)
time and rate of improvement of diarrhea, myalgia, fatigue and other symptoms
time and rate of pulmonary imaging improvement

whether patients had negative conversion of SARS-CoV-2 by 28 days and whether patients with severe covid-19 had clinical improvement by 28 days.

Viral nucleic acid test

primary safety data (infusional and allergic reactions, secondary infection and life-threatening adverse events) and the primary efficacy data (the level of the cytokines variation, the level of C-reactive protein in plasma and the oxygen saturation).

the median time of hospital stay

Improved respiratory system function (blood oxygen saturation) recovery time

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the time to clinical improvement, ie, the duration fromrandomization to the improvement of at least 1 point ona 7-category ordinal scale (adopted from a scale for hospitalized patients with severe influenza) or discharge from hospital, whichever occurred first

Clinical symptoms

Blood routine

the viral load of 2019-nCOV of throat swab

TBNK cell subsets

TH1/TH2 Cytokine

the time interval from admission to viral RNA negative conversion, patients were admitted to hospital on the day or the following day (<24 hours) after the diagnosis of the first positive RNA test

viral negative-transforming time

30-day cause-specific mortality

30-day cause-adverse events

30-day all-cause mortality

co-infections

Not specified

Complete Blood Count

CRP

blood coagulation

D-dimer

Virological examination of pharyngeal swab

the time from Leflunomide initiation to clinical improvement meeting the discharge criteria (set by the National Health Commission of China)

duration of viral shedding, which was defined as the time from randomization to the first negative nucleic acid test of five consecutive RT-PCR results.

The days from positive to negative for viral nucleic acid testing

HAMD17 and HAMA

the score of Hamilton depression scale

the score of Hamilton anxiety scale

the score of Self-Rating Depression Scale

the score of Self-Rating Anxiety Scale

the level of peripheral blood lymphocyte and IL-6 on the 1st, 7th, 14th day of the enrollment,

The percentage of patients who convert to moderate one
The rate of shock
Endotracheal intubation ratio
Time spent on the ventilator
mortality

the cumulative lung lesion remission rate (lung CT examination indicated absorption of lung inflammation).

Clinical cure rate

Phase 1:
Safety endpoints:
z The primary safety endpoint is the incidence of adverse reactions within 7 days after each injection;
primary safety outcome
The 7-day adverse reactions
primary humoral immunogenicity outcomes
neutralizing antibody titres and specific IgG binding antibody titres measured on

Incidence of adverse reactions/events

the occurrence of adverse reactions within 7 days after the first and second vaccinations.

the occurrence of adverse reactions within 7 days after each vaccination

Incidence of adverse reactions/events

the incidence of solicited local reactions at the injection site or systemic adverse reactions in the 14 d after the prime or boost dose, and adverse events after the full immunization in the 28 d after receiving the boost dose. Any clinical laboratory abnormalities from baseline to 24 h or 7 d after the prime or boost.
Occurrence of solicited local reactions in the subjects (e.g., vaccination sites: pain/tenderness, erythema/redness, induration/swelling) during the 14-days after each dose of BNT162b1 or placebo.
Occurrence of solicited systematic reactions (e.g., nausea, vomiting, diarrhea, headache, fatigue, myalgia, arthralgia, chills, loss of appetite, malaise, and
Phase I
Incidence of adverse events within 28 d after each dose of vaccine.

Phase II

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Incidence of adverse reactions/events

The seropositive rates and level of SARS-CoV-2 neutralizing antibody
The seropositive rates of SARS-CoV-2 IgG antibody (tested by ELISA)

The primary endpoints for immunogenicity were neutralization antibody seroconversion and titre, and RBD-IgG seroconversion 28 days after the third dose.

The seropositive rates and level of SARS-CoV-2 neutralizing antibody
The seropositive rates of SARS-CoV-2 IgG antibody (tested by ELISA)

depression, anxiety and stress levels using the Chinese Version of Depression Anxiety and Stress Scale-21 (DASS-21)

Psychological health level of the patient with COVID-19, assessed using the Chinese Version of Depression Anxiety and Stress Scale-21 (DASS-21) for all participants at baseline and post- intervention (1 day later)

the incidents of ARDS development;

The incidents of acute respiratory distress syndrome (ARDS) development

viral clearance rate at day 7 after randomization

The virological clearance rate of throat swabs, sputum, or lower respiratory tract secretions at day 7

the rate of positive-to-negative conversion of SARS-CoV-2 nucleic acid from the initiation of treatment to day 21, with the enrollment day as the first day of treatment.

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The rate of virus inhibition

time to clinical improvement within 28 days after randomisation.

Time to Clinical Improvement (TTCI)

主要研究终点为第7天咽拭子、痰液或下呼吸道分泌物等样本病毒学清除或2周内患者发生死亡

The primary endpoint was negative conversion rate of SARS-CoV-2 nucleic acid in respiratory pharyngeal swab on days 7 after randomization.

The virological clearance rate of throat swabs, sputum, or lower respiratory tract secretions at day 3

The virological clearance rate of throat swabs, sputum, or lower respiratory tract secretions at day 5

The virological clearance rate of throat swabs, sputum, or lower respiratory tract

invasive mechanical ventilation (IMV)-free days in 28 days (IMVFD28).

Ventilation-free days

the incidence of clinical deterioration 14 days after randomization.

the incidence of treatment failure in 14 days

The primary end points were the time to clinical recovery and the deterioration rate after initiation of medications.

Time to clinical recovery after treatment

Rate of aggravation

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change in the total lesion proportion (%) of the whole lung volume from baseline to day 28, as measured by chest CT. It was defined as (total lesion proportion of the whole lung volume at day 28–total lesion proportion of the whole lung volume at baseline)/total lesion proportion of the whole lung volume at baseline

change in the total lesion proportion (%) of the whole lung volume from baseline to month 1

Change in lesion proportion (%) of full lung volume from baseline to day 28.

The primary endpoint for safety evaluation was the incidence of adverse reactions within 14 days after the injection. The primary endpoints for immunogenicity were the geometric mean titres (GMTs) of RBD-specific ELISA antibody responses and neutralising antibody responses against live virus or

Occurrence of adverse reactions
Anti SARS-CoV-2 S IgG antibody response(ELISA)
Neutralizing antibody response to SARS-CoV-2

Phase I
Incidence of adverse reactions occur from the beginning of the vaccination to 28 days after the whole schedule vaccination
Phase II

Safety indexes of adverse reactions
Immunogenicity indexes of neutralizing-antibody seroconversion rates

The primary safety endpoint was any vaccine-related adverse event (adverse reaction) within 28 days after the administration of each dose of vaccine or placebo. The primary immunogenicity endpoint was the seroconversion rate of neutralising antibodies to live SARS-CoV-2 at day 28 after the second dose.

Safety index-incidence of adverse reactions
Immunogenicity index-seroconversion rates of neutralizing antibody

Phase 2 trial among adults aged 18-59 years old
Primary endpoint
- Positive conversion rate of serum neutralizing antibody on Day 14 (Cohort 1)/Day 28 (Cohort 2) after two doses of test vaccine;

Safety indexes of adverse reactions
Immunogenicity indexes of neutralizing-antibody seroconversion rates

Safety index-incidence of adverse reactions
Immunogenicity index-seroconversion rates of neutralizing antibody

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the seroconversion rates of the anti–SARS-CoV-2 neutralizing antibody and ELISA immunoglobulin G (IgG) antibody at day 14 (0, 14 procedure) and day 28 (0, 28 procedure), respectively, after the boost immunization.

The primary end point was the total rate of adverse reactions from 0 to 28 days post-immunization.

Adverse reactions/events rate

Adverse reactions/events rate

Seroconversion rate of Neutralizing antibodies against SARS-CoV-2 Phase IIa (Day 0, 14 schedule)

Seroconversion rate of Neutralizing antibodies against SARS-CoV-2 Phase IIa

Not specified

Correlation of adverse events with the investigational product

For the phase 1 trial, the primary outcome was the safety of the COVID-19 vaccine. For the phase 2 trial, primary outcomes were safety and immunogenicity.

Safety outcomes were the occurrence of adverse events between the first

The number of adverse events after intramuscular injection

Neutralizing antibody positive conversion rate

Phase 1 trial: the incidence of adverse reactions within 7 days after each dose vaccination

Phase 2 trial: the GMT of RBD-specific ELISA antibody responses at 30 days after the last dose vaccination and the incidence of adverse reactions within 7

Occurrence of adverse reactions (AR) within 7 days after each dose of the recombinant SARS-CoV-2 vaccine (Sf9 cell) or placebo.

Geometric mean (GMT) of specific antibody

The incidence of adverse reaction (AR)

the percentage of subjects with viral negative by Day 14

the time from randomization to clinical improvement

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Time to viral negativity by RT-PCR
Time to clinical improvement

symptom improvement rate and symptom disappearance rate after 14 days of treatment;

clinical symptom improvement and disappearance rates after 14 days of treatment

the negative conversion ratio
the proportion of general patients with advanced severe disease
confirmed rate of suspected cases

the rate of worsening before clinical endpoint occurrence

Rate of conversion to severe or critical illness

clinical improvement

14 day outcome of the subjects, including: recovery, improvement, turning critical, death.
lung CT

SOFA Score

SOFA

The primary outcome in the study of moderate COVID-19 was time to clinical recovery.
The primary outcome in the study of severe COVID-19 was time to clinical improvement.

TTCI (Time to Clinical Improvement)

Time to Clinical Recovery, TTCR

modified Medical Research Council (mMRC) dyspnea scale

lung function
ADL
6min walk

The time required for a negative SARS-CoV-2 RNA result or the rate of negative results on day 10.

COVID-19 nucleic acid detection time from positive to negative (respiratory secretion) or (3,5,7,10 days from positive to negative rate)
Lung CT observation of inflammation absorption
Proportion of cases with progressive disease

the conversion time of the SARS-CoV-2 viral assay

The time to 2019-nCoV RNA negativity in patients

the difference between carbon dioxide (CO2) concentration in the examination room at baseline and the peak value during examination
adenosine triphosphate (ATP) fluorescence value

During the operation, the patient's volume of local exhaled air from the mouth and nose, the patient's heart rate, respiratory rate and blood oxygen saturation

退热时间

fever clearance time
Effective rate of TCM symptoms

PaO2/FiO2
ROX index (ROX=SpO2/(FiO2*RR))

PO2/FiO2
ROX INDEX

the rate of symptom recovery after 14 days of treatment

The time for the positive nucleic acid of the novel coronavirus to turn negative
Incidence of deterioration
The defervescence time
Chest CT

6 min walking distance (6MWD) in metres.

Total distances of 6 minutes walk test

improvements in 6MWD and findings on chest computed tomography (CT) images.

St George's respiratory questionnaire score
Fatigue assessment scale
Six minute walking experiment

incidence of adverse events or adverse reactions within 60 min, and at days 7, 14, and 28 after each vaccine dose.

Specific IgG antibody
Neutralizing antibody
Cellular immunity

PTSS of COVID-19 patients accessed before and after intervention

Photo - C grade
SAS
SDS
PSQI

The primary immunogenic endpoints were the seroconversion rates [geometric mean titer (GMT) ≥ 16] and GMT of neutralizing antibody to live SARS-CoV-2 at day 28 after the last dose.
The primary endpoint for safety was the occurrence of adverse reactions within

Serum neutralizing antibodies, IgG antibodies, and IgM antibodies of SARS-CoV-2 28 days after the last dose of vaccination

Serum neutralizing antibodies, IgG antibodies, and IgM antibodies of SARS-CoV-2 28 days after the last dose of vaccination

The primary endpoint was the rate of positive nucleic acid tests performed on nasal and pharyngeal swabs during the quarantine medical observation period.

Nucleic acid test positive rate of nasal and throat swabs during the isolation medical observation period

The primary immunogenicity outcomes were the seroconversion rate, geometric mean titer (GMT) of the RBDbinding antibody and SARS-CoV-2 neutralizing antibody.

1. Positive conversion rate of serum anti-SARS-CoV-2 RBD protein antibody, and its geometric mean titer (GMT) and geometric mean fold increase (GMI)
2. Positive conversion rate of serum anti-SARS-CoV-2 neutralizing antibody, and its GMI

The safety outcomes were the frequencies and percentages of AEs within seven days after the vaccination, AEs between the first vaccination to 30 days after the final vaccination, any abnormal changes in laboratory measures at seven days post-vaccination, and SAEs during the whole follow-up period. AEs,

Safety

time to achieve a consecutive twice (at intervals of more than 24 h) negative RT-PCR result for SARS-CoV-2 RNA in nasopharyngeal swab and sputum sample.

Viral nucleic acid test negative conversion rate

DLT and MTD of SCTA01

Dose-limiting toxicity(DLT)
Maximal Tolerable Dose(MTD)

the safety and tolerability of MW33

Adverse Event
Serious Adverse Event

SARS-CoV-2 viral load

Primary Endpoint
- Incidence rate of adverse reactions occured from the beginning of the vaccination to 28 days after the second dose vaccination
Primary Endpoint

Safety index-incidence of adverse reactions
Immunogenicity index-seroconversion rates of neutralizing antibody

the incidence of ARs within 14 days after each vaccination and the GMTs of RBD-specific ELISA antibodies and pseudovirus neutralizing antibodies on day 28 after boost vaccination

Safety indexes of adverse reactions
Immunogencity indexes of GMT
Immunogencity indexes of neutralizing antibody

The occurrence of adverse reactions within 28 d after boosting was evaluated as the primary endpoint for safety
The GMT of neutralizing antibodies against live SARS-CoV-2 virus at 14 d after boosting was the primary immunogenic endpoint.

Incidence of adverse reactions within 28 days after the booster dose.
GMT of neutralizing antibodies against live SARS-CoV-2 virus on day 14 after the booster dose.

Not specified

Treatment-emergent Changes in Body Temperature
Treatment-emergent Changes in Respiratory Rate
Treatment-emergent Changes in Pulse Rate
Treatment-emergent Changes in Blood Pressure

Not specified

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Immunogenicity of Third Dose Vaccine Booster Following Two Doses of Inactivated Vaccines

Reactogenicity of Third Dose Vaccine Booster Following Two Doses of Inactivated Vaccines

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Secondary Outcome

Other clinical outcomes included clinical status as assessed with the seven-category ordinal scale on days 7 and 14, mortality at day 28, the duration of mechanical ventilation, the duration of hospitalization in survivors, and the time to clinical improvement (TTCI)

Proportion of patients in each category of the 7-point scale on days 7, 14, and 28 after randomization;

mortality on day 28

Duration of mechanical ventilation(days)

the differences among the three groups in the proportion of patients with SARS-CoV-2 nucleic acid negativity at day 14,

the mortality rate at day 28,

mortality on day 28

The rate of negative 2019-nCoV RNA results at day 14

the rate of aggravation

the rate of adverse events

The rate of discontinuations due to adverse events

the overall survival through last day of treatment, the proportion of patients without improvement (scored 3–5 on the COVID-19 severity scale), the change in serum procalcitonin level after treatment, and the prevalence of antibiotic use during treatment

Oxygenation index

PCT

Antibiotic use

the median time to SARS-CoV-2 clearance.

time till the SARS-CoV-2 clearance

the proportion of patients who achieved clinical symptom recovery on days 7 and 10,

the time for clinical recovery of the patients,

the time for nucleic acid turning negative (defined as two consecutive

clinical symptom disappearance time

Time and rate of virus turning negative

Length of hospital stay

Antipyretic time

Severe conversion rate

the time to radiological improvement defined as the time from enrolment to radiological improvement on chest CT scans,

the time to virus nucleic acid negative conversion defined as the time from enrolment to two consecutive negative tests for SARS-CoV-2 via RT-PCR

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time to discharge
mortality
deterioration rate
adverse events

the lowest SpO2 during intubation, incidence of mask ventilation for SpO2 <90%, PaO2/FIO2 before intubation, incidence of SpO2 <80% during intubation, incidence of minimum SpO2 >95% during intubation, and 7 day mortality

the incidence of hypoxia
the times of intubation
Jaw thrust maneuver
Length of stay in ICU
Death

(1) 28-day mortality, including analysis of time from randomization to death; (2) duration of hospitalization, including analyses of time from randomization to discharge, time from admission to discharge, and 28-day discharge rates; and

28-day mortality
Hospitalization time
ICU hospitalization
Invasive mechanical ventilation during infection
ECMO duration during infection

Secondary outcomes were the improvement rate of chest CT images, the time required for the body temperature returning to normal and the improvement in respiratory symptoms and signs.

#BLANK

Probability of negative conversion at day 4, 7, 10, 14, or 21 was specified as a secondary outcome in the protocol, but this does not appear on the trial registration list. Other prespecified secondary outcomes not listed in the trial registration but included in the protocol

#BLANK

The secondary efficacy outcomes mainly included the total lymphocyte count and subpopulations, the chest CT, the respiratory rate, and the patient symptoms (especially the fever and shortness of breath). In addition, the therapeutic measures (i.e. antiviral medicine and respiratory support) and

the levels of serum C-reactive protein (CRP)

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Secondary outcomes included lymphocyte cell counts on day 5 posttreatment, mortality at day 21, the proportions of patients progressing to critical conditions (eg, acute respiratory distress syndrome, sepsis, or septic shock), and viral loads (reflected by the Ct values as measured with real-time

#BLANK

Secondary outcomes included chest computed tomography (CT) improvement time, symptom improvement/disappearance time, viral shedding duration, length of hospital stay, mortality, and complications during treatment. Patients were followed up to 28 days after discharge. Other

#BLANK

Not specified

#BLANK

Other clinical outcomes included length of hospitalization, duration of viral shedding, and safety outcomes.

Other clinical outcomes included clinical status, i.e. progressive rate to severe illness, syndromes, peripheral blood cells and biochemical parameters, C-reactive protein and inflammatory cytokines, length of hospital stay, etc. Safety outcomes included adverse events that occurred during

Rate of progress to severe illness
Duration of fever
C reactive protein concentration in peripheral blood
Absolute value and percentage of lymphocytes in peripheral blood

Secondary end points for evaluating the efficacy of cCBT were SDS to assess self-rated depressive symptoms, SAS to assess anxiety symptoms, and AIS to assess insomnia symptoms.

#BLANK

(1) 28-day follow-up mortality, the rate of mechanical ventilation, septic shock and conversion to the moderate type during 14 days of treatment.
(2) Peripheral blood leukocytes, C-reactive protein (CRP), IL-8 and TNF-α on the 1st, 7th, 14th day of the enrollment. Sample collection was carried out

CRP
IL-6
TNF-α
Time of improvement of clinical symptoms (fever, cough)
Adverse drug reactions

the improvement of clinical symptoms (cough, diarrhea, dyspnea, fever, myalgia) before and after treatment;
the changes of blood routine test and IL-6;
Viral nucleic acid test negative conversion rate and days from positive to negative
Duration of fever
Lung imaging improvement time
Mortality rate because of Corona Virus Disease 2019

Phase 1:
The secondary safety endpoints include:
z Incidence of any adverse reactions/events within 30 minutes after each injection;
Secondary immunogenicity outcomes
neutralizing antibody titres and specific IgG binding antibody titres

Secondary safety outcome
Incidence of abnormal indicators of liver and kidney function, blood routine and urine routine
Rate of adverse reaction and adverse event
Incidence of Serious Adverse Events (SAE)

Any abnormal changes in laboratory measures at day 4 post inoculations, and adverse events within 28 days after the first and the second vaccinations across the treatment groups were analysed as secondary safety endpoints.
The secondary humoral immunogenicity outcomes were measured using an adverse reactions within 30 days after whole vaccination procedure across all study groups were analysed as secondary safety endpoints.
The secondary outcomes for humoral immunogenicity were measured with infectious SARS-CoV-2 neutralising assay (on days 0, 28, 56, and 84) and
Incidence of abnormal indicators of liver and kidney function, blood routine and urine routine
Incidence of adverse reactions/events
Incidence of Serious Adverse Events (SAE)

Four-fold growth rate and antibody level (GMT, GMI) of serum antibody
The secondary endpoints for immunogenicity were GMT, seroconversion rate and fold increase of virus-neutralizing antibody, or ELISA IgG antibodies binding to S1 or RBD measured 7 d and 21 d after both the prime dose and the boost dose.
The proportion of subjects experiencing serious adverse events (SAEs), occurring up to Day 21 after prime vaccination and Day 28 after boost vaccination, up to Month 3, 6 and 12.
The proportion of subjects experiencing AE associated with BNT162b1, occurring up to Month 3, 6 and 12.

Phase I
Incidence of serious adverse events from the first dose to 12 months after full-course immunization.
Incidence of abnormal blood routine, blood biochemistry, coagulation

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Serious Adverse Events(SAE)
Abnormal indicators of blood routine, blood biochemistry, urine routine and coagulation function
The seropositive rates and level of SARS-CoV-2 neutralizing antibody and IgG antibody(tested by ELISA)

Incidence of adverse reactions/events
Serious Adverse Events (SAE)
Level of SARS-CoV-2 IgG antibody (tested by ELISA)
The seropositive rates and level of SARS-CoV-2 neutralizing antibody and IgG antibody (tested by ELISA)

proportion of participants experiencing adverse reactions/events within 28 days following each vaccination,
occurrence of SAE from the first dose through 12 months post the third dose,
titre of RBD-IgG 28 days after the third dose,

Incidence of adverse reactions/events
Serious Adverse Events (SAE)
Level of SARS-CoV-2 IgG antibody (tested by ELISA)
The seropositive rates and level of SARS-CoV-2 neutralizing antibody and

Not specified

#BLANK

the time to fever resolution
the recovery of lung injury

The time to fever resolution rate
Time to recovery of lung injury

viral clearance at day 3 and day 5,
the critical illness rate of subjects during the 14 days after randomization,
the mortality rate of subjects at day 14, and
the number of participants with treatment-related adverse events

The virological clearance rate of throat swabs, sputum, or lower respiratory tract secretions at day 3
The virological clearance rate of throat swabs, sputum, or lower respiratory tract secretions at day 5
Number of participants with treatment-related adverse events as assessed

1) the rate of positive-to-negative conversion of SARS-CoV-2 nucleic acid at day 14;
2) the rate of antipyresis (defined as axillary temperature ≤ 37.3°C for more than 72 hours) from the first day of treatment;

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The disease prorogation-temperature
The disease prorogation-respiratory function 1
The disease prorogation-respiratory function 2
The disease prorogation-respiratory function 3

the proportions of patients in each category of the six-point scale at day 7, 14, and 28 after randomisation;
all-cause mortality at day 28;
frequency of invasive mechanical ventilation;

Clinical status
Time to Hospital Discharge OR NEWS2 (National Early Warning Score 2) of ≤ 2 maintained for 24 hours.
All cause mortality
Duration (days) of mechanical ventilation

次要研究终点为2周内发生严重药物不良反应或受试者病情转为重型、危重型

Number of participants with treatment-related adverse events as assessed by CTCAE v5.0
The critical illness rate of subjects at weeks 2

28-day mortality,
organ functions and inflammatory parameters, including white blood cell counts, neutrophil counts, lymphocyte counts, procalcitonin, interleukin-6 (IL-6), and C-reactive protein (CRP).

28-days mortality
ICU length of stay
Demand for first aid measuments
Vasopressor days
Respiratory indexes

the incidence of clinical cure 14 days after randomization,
the incidence of intensive care unit (ICU) admission,
in-hospital mortality,
the time from randomization to clinical cure

clinical cure incidence in 14 days
the duration of virus change to negative
mortality at day 30
ICU admission rate in 30 days

The secondary outcomes assessed the virologic clearance during the study period (SARS-CoV-2-negative conversion within 20 days, rate of SARS-CoV-2-negative conversion), clinical follow-up (clinical remission rate, time to fever remission, rate of significant improvement in chest CT, percentage of

Clinical remission rate
Dynamic changes of oxygenation index
Time to cure
rate to cure
Time to defervescence

The secondary imaging outcomes were a change in the total lesion proportion (%) of the whole lung volume from baseline to day 10, a change in solid component and ground-glass lesion proportion from baseline to day 10, 28, and change in lung densitometry at day 10, 28. Clinical outcomes within

change in the total or solid component lesion proportion (%) of the whole lung volume from baseline to followup

Change in lesion proportion (%) of full lung volume from baseline to day 10 and 90
Change in consolidation lesion proportion (%) of full lung volume from baseline to day 10, 28 and 90.

The secondary endpoints for immunogenicity were RBD-specific ELISA antibody responses at day 14 and month 6 (6 month data not yet available), and specific T-cell responses at day 28 post vaccination. Seroconversion of the humoral immune responses was also a secondary endpoint, and was

Occurrence of adverse events
Occurrence of serious adverse reaction
Anti SARS-CoV-2 S IgG antibody response(ELISA)
Neutralizing antibody response to SARS-CoV-2
Neutralizing antibody response to Ad5-vector

Phase I
Incidence of adverse reactions within 7 days after each dose of vaccination ;
Incidence of abnormal laboratory index (blood routine test, blood chemistry

Safety indexes of adverse reactions
Safety indexes of Incidence of abnormal laboratory index
Safety indexes of serious adverse events (SAEs)
Immunogenicity indexes of the seropositive rate, GMT, and GMI of neutralizing antibodies

Secondary endpoints were severe adverse events and geometric mean titre (GMT) of neutralising antibodies to live SARS-CoV-2, as well as seropositive rates and geometric mean increase.

Safety index-incidence rate of adverse reactions
Safety index-incidence rate of serious adverse events in phase Ⅰ
Safety index-incidence rate of serious adverse events in phase Ⅱ
Immunogenicity index-seropositive rate, GMT, and GMI of neutralizing antibodies

Secondary endpoint
- Positive rate, GMT and GMI of serum neutralizing antibody on Day 28 after two doses of test vaccine;
- Positive conversion rate, positive rate, GMT and GMI of serum neutralizing

Safety indexes of adverse reactions
Safety indexes of Incidence of abnormal laboratory index
Safety indexes of serious adverse events (SAEs)
Immunogenicity indexes of the seropositive rate, GMT, and GMI of

Safety index-incidence rate of adverse reactions
Safety index-incidence rate of serious adverse events in phase Ⅰ
Safety index-incidence rate of serious adverse events in phase Ⅱ
Immunogenicity index-seropositive rate, GMT, and GMI of neutralizing

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(1) the total AE rate from 0 to 7 days following each inoculation and from 0 to 28 days after the boost immunization,
(2) the geometric mean titer (GMT) profiles of the anti-SARS-CoV-2 neutralizing antibody and ELISA IgG antibody at days 14 and 28 (0, 14

The secondary end points were serological evidence of the immunogenicity of the vaccine.

Serious adverse events
Level of Neutralizing antibodies against SARS-CoV-2 Phase Ia (Day 0, 14 schedule)
Level of Neutralizing antibodies against SARS-CoV-2 Phase IIa (Day 0, 14 schedule)

Not specified

Primary pharmacokinetic variables
Primary pharmacokinetic variables
Primary pharmacokinetic variables
Primary pharmacokinetic variables

For the phase 1 trial, the secondary outcome was immunogenicity. Outcomes for immunogenicity were the seroconversion rate and magnitude, in geometric mean titres (GMTs), of the RBD-binding antibody, SARS-CoV-2 neutralising antibody, and T-cell cytokine production (primary for phase II)

Immunogenic end point

Neutralizing antibody GMT, positive rate
Neutralizing antibody GMT, positive conversion rate/positive rate
Neutralizing antibody GMT
Neutralizing antibody GMT
Neutralizing antibody GMT, positive rate

The secondary endpoints in phase 1 trial for safety were the incidence of adverse events and serious adverse events from the first dose to 28 days after the last dose, and abnormal changes in laboratory measures at 3 days after each dose vaccination.
Occurrence of adverse events (AE) within 7 days after each dose of the recombinant SARS-CoV-2 vaccine (Sf9 cell) or placebo.
Occurrence of AE up to Day 28 after prime and boost vaccination.
The proportion of SAEs up to Day 28 after prime and boost vaccination.
The incidence of adverse events (AE)
The incidence of severe adverse events (SAE)
The incidence of serious adverse events
Geometric mean (GMT) of anti-SARS-CoV-2-specific neutralizing antibodies
the percentage of subjects with viral negative by Day 7
the incidence of mechanical ventilation by Day 14
ICU admission by Day 14
all-cause mortality by Day 14

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Incidence of mechanical ventilation by day14
Incidence of ICU admission by day14
Time to treatment failure
All-cause mortality by day14, day28

the disease aggravation rate after 14 days of treatment

the proportion of patients who progressed to severe status despite the same duration of treatment

Traditional Chinese medical syndrome scale
Body temperature

discharge with cure
composite symptom outcome

Rate of disisolation
TCM symptom
Body temperature
Blood routine test

mortality
rate of advancing to critical category
receiving an invasive ventilator in the severe category
vital signs
oxygenation index
Routine Blood
inflammatory biomarkers

all-cause mortality up to 30 days

cure rate
All-cause mortality
Severity rate

The secondary outcomes in the study of moderate COVID-19 included:
(1) 28-day mortality,
(2) time to virological clearance,
(3) frequency of dyspnea progression,
Proportion of subjects with 6-grades rating form at fixed time points (days 7, 14, 21, and 28 after randomization)
All-cause mortality of 28-days
Duration of mechanical ventilation (days) of 28-days

All-cause mortality of 28-days
Transition time of 2019-nCoV nucleic acid detection
Frequency of respiratory progression：Defined as SPO2≤ 94% on room air or PaO2/FiO2 <300mmHg and requirement for supplemental oxygen or more
the modified Borg dyspnea scale (MBS)
fatigue Scale-14 (FS-14)
patient health questionnaire-9 scale (PHQ-9)
duration of respiratory symptoms
PHQ-9
Length of hospital stay
Respiratory symptoms
immune cells

The improvement rate and time of cardinal symptoms (axillary temperature, transcutaneous oxygen saturation, and respiratory frequency recovery)

#BLANK

1) the change in the 7-point scale;
2) the rate of critical aggravation;
3) blood routine test outcomes;
4) blood biochemical test outcomes;
The 7-point scale
The rate of critcal aggravation
Blood routine test outcomes
Blood biochemical test outcomes

adverse events
COVID-19
examination time

New coronavirus infection before and after treatment, and new coronavirus infection among relevant medical staff (including chest CT, viral nucleic acid detection, clinical diagnosis, etc.)

入院第 1 天及研究第 7 天，记录单项症状消失情况

body tempture
lung CT scan
clearance time of COVID-19
Oxygen saturation

the white blood cell (WBC) count
peripheral blood mononuclear lymphocyte (PBML) count
respiratory support step-down time (RSST)

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WBC
PBML
Time of Support time

the time to recovery of individual symptoms,
TCM syndrome scores,
the time of conversion of SARS-CoV-2 RNA assay,
hospitalization days,
Haematological
C-reactive protein
Blood gas analysis
Urine test

squat time in seconds;
pulmonary function assessed by spirometry;
HRQOL measured with Short Form Health Survey-12 (SF-12)
mMRC-dyspnoea
pulmonary function
mMRC dyspnea scale
strength of lower limber muscles
ICF-COVID simple evaluation form

Chinese medicine syndrome scores,
St. George's Respiratory Questionnaire (SGRQ) score,
Borg-Dyspnea Scale scores,
Fatigue Assessment Inventory (FAI) scores

#BLANK

The secondary endpoint for safety was abnormal changes in laboratory tests at days 1, 4, 7, and 28 after each vaccine dose.

For immunogenicity, the secondary endpoint was titres of neutralising

#BLANK

depression, anxiety, and sleep quality

#BLANK

Secondary immunogenic endpoints were the positive rates (GMT ≥ 32, 64, 128, 256) 28 days after the whole course of vaccination, respectively. Adverse events within 28 days after the first and the second vaccinations across the three groups were analyzed as secondary safety endpoints.

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The specific immune cell level of SARS-CoV-2 in whole blood 28 days after the last dose of vaccination

Serum neutralizing antibodies, IgG antibodies, and IgM antibodies of SARS-CoV-2 after enhancing immunity of vaccination

The specific immune cell level of SARS-CoV-2 in whole blood 28 days after the last dose of vaccination

Serum neutralizing antibodies, IgG antibodies, and IgM antibodies of SARS-CoV-2 after enhancing immunity of vaccination

the time of the occurrence of positive nucleic acid tests of nasal and pharyngeal swabs during the quarantine medical observation period;

the proportion of asymptomatic people with positive nucleic acid tests;

the proportion of mild, medium, and severe patients with positive nucleic acid

Time of occurrence of positive nucleic acid tests in nasal and throat swabs during the isolation medical observation period

Proportion of asymptomatic infections among positive nucleic acid tests

Proportion of mild, normal, and severe patients with positive nucleic acid

Not specified

Safety outcomes

Immunogenicity outcomes

Not specified

Immunogenicity

the changes of blood routine and CRP (C-reactive protein);

the count and proportion of T lymphocyte subsets in peripheral blood and the changes of cytokines

Clinical cure rate

The secondary endpoints were the PK parameters, including the integral of the concentration-time curve from dosing to time point (AUC0–t), the integral of the concentration-time curve from dosing to infinity (AUC0–∞), half-life (t1/2), the time taken to reach the maximum concentration (Tmax),

AUC0-t

AUC0-∞

t1/2

Tmax

PK and assessment of ADAs to MW33

#BLANK

#BLANK

Secondary Endpoints

- Incidence rate of adverse reactions within 7 days after each dose vaccination;

- Incidence of abnormal laboratory index (blood routine test, blood chemistry

Safety index-incidence of serious adverse events

Immunogenicity index-seropositive rates of neutralizing antibody

Immunogenicity index-geometric mean titer (GMT) of neutralizing antibody

Immunogenicity index-geometric mean ratio (GMR) of neutralizing antibody

adverse events within 28 days after each vaccination,

serious adverse events reported up to 6 months,

any abnormal changes in laboratory measures at 4 days after each vaccination for sentinel participants,

Safety indexes of adverse events

Safety indexes of Hematological examination measures (Hemoglobin, WBC)

Safety indexes of Blood routine measures(ALT, AST)

Incidence of solicited adverse reactions within 14 d,

unsolicited adverse reactions within 28 d after boosting

serious adverse events reported during the 6-month study period

eceptor-binding domain (RBD)- and nucleocapsid (N)-specific

Incidence of solicited AE within 14 days after the booster dose

Incidence of unsolicited AE within 28 days after the booster dose.

Incidence of serious adverse events (SAE) till the 6 months after the booster dose.

Not specified

Assess Pharmacokinetic parameters of LY-CovMab

Assess Pharmacokinetic parameters of LY-CovMab

Assess Pharmacokinetic parameters of LY-CovMab

Assess Pharmacokinetic parameters of LY-CovMab

Not specified

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The seropositive rate of neutralizing antibodies
GMT of neutralizing antibodies
GMFR of neutralizing antibodies
severe adverse events and adverse events of special interest

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New primary outcome introduced in publication (1 = Yes, 0 = No)	Original primary outcome omitted in publication (1 = Yes, 0 = No)
1	1
0	1

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0	0
0	1
1	0
1	1
1	1

For peer review only

1	1
0	1
1	0
0	0
0	1

peer review

1	1
0	1
0	0
0	1

eview only

peer review only

1 1

For peer review

For peer review only

1 0

1 1

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1	1
1	1
0	1

For peer review only

1	1
0	1
1	1
0	0

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0	1
1	0
1	1
1	1
0	0

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1	0
0	0

For peer review only



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For peer review only

Secondary outcome in registry described as primary outcome in publication (1 = Yes, 0 = No)	Primary outcome in registry described as secondary outcome in publication (1 = Yes, 0 = No)
0	0
0	1

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1	1
0	1
0	0
0	0
0	0

For peer review only

0	1
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0	1
0	1
1	1

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1	0

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0 0

0 0

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0	0
0	1

0	1

For peer review only

0	0
0	0
0	0
0	1

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0	1
0	1
0	1
0	1
1	0

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0	0
1	0

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Deviation in time of Primary outcome Measurement(1 if Yes, 0 if not specified in one or more of data sources)
0

For peer review only

For peer review only

For peer review only

For peer review only

0

1

1

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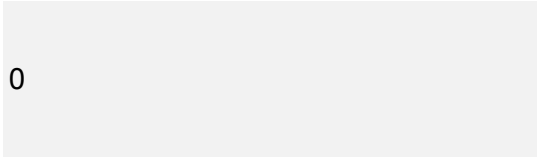
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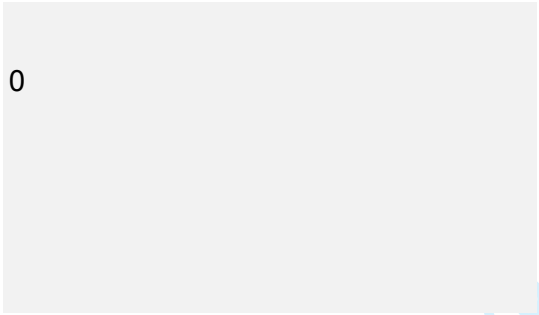
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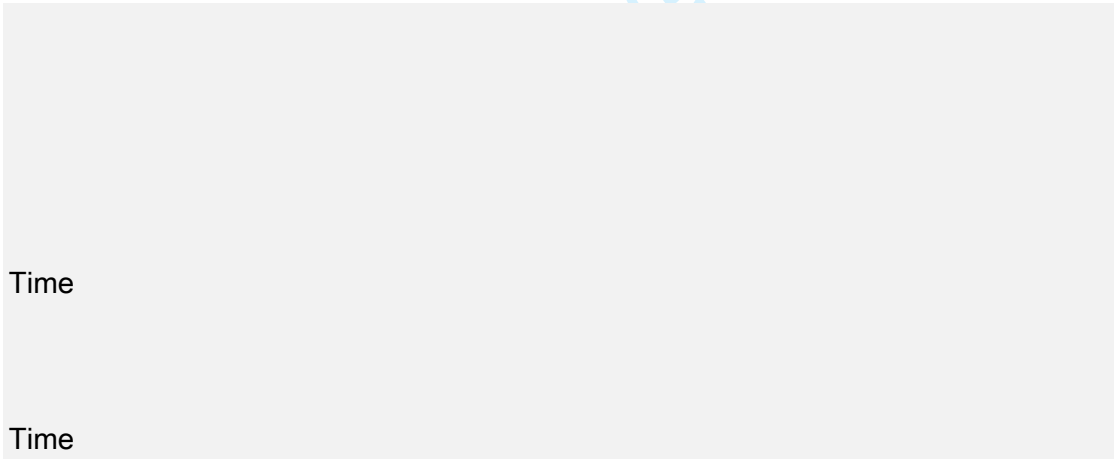
Time of Primary Outcome Measurement
Time
Time
Time
Time
day six of antiviral treatment
day 6 after treatment
on day 14 of treatment
Not Specified

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Time

Time

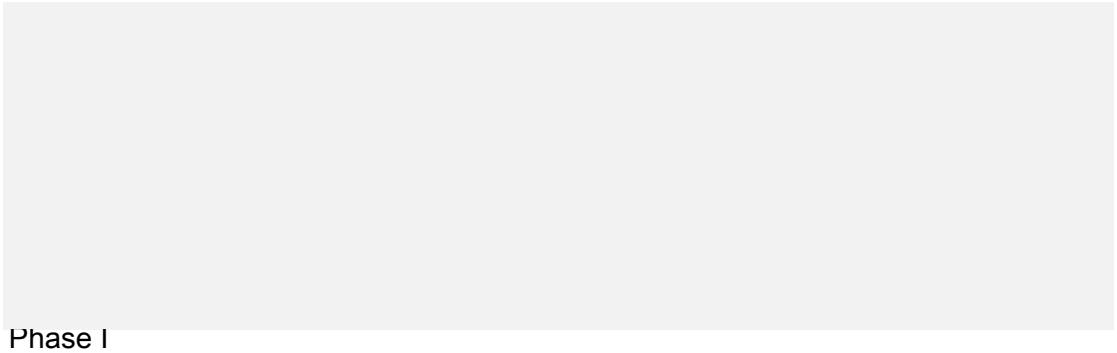
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Time
Time

within 7 days after the first and second vaccination

within 7 days after each vaccination

0-7 days after each dose of vaccination



Phase I
within 28 d after each dose of vaccine
Phase II
28 days after the whole immunization

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0-28 days after each dose of vaccination

Day 28 post full vaccination

28 days after the third dose

Day 28 post full vaccination
Day 28 post full vaccination

Not Specified

1 day later

28 days

Time Frame: 14 days

day 7 after randomization

at day 7

from the initiation of treatment to day 21

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Time Frame: Day 0, 2, 4, 7, 10, 14 and 21

Time

Time Frame: up to 28 days

Day7

Day3, Day5, Day7

14 days after randomization

in 14 days

within 14 days

Time Frame: within 14 days from the start of medication
Time Frame: within 14 days from the start of medication

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from baseline to day 28

month 1

Time Frame: Day 28

within 14 days after the injection
at day 28 post vaccination
at day 28 post vaccination

Time Frame: 0-14 days post vaccination
Time Frame: 28 days post vaccination
Time Frame: 28 days post vaccination

from the beginning of the vaccination to 28 days after the whole schedule vaccination
on the 14th (for the emergency immunization schedule) or 28th (for the routine immunization schedule) day after the whole schedule vaccination

Time Frame: From Day 0 to Day 28 after each dose
Time Frame: The 14th day /the 28th day after two doses of vaccination

Phase 2 trial among adults aged 18-59 years old

on Day 14 (Cohort 1)/Day 28 (Cohort 2) after two doses of test vaccine;
on Day 0-28 (Day 0-14 for the first dose for Cohort 1) after each dose;

Time Frame: From Day 0 to Day 28 after each dose
Time Frame: The 14th day /the 28th day after two doses of vaccination

Time Frame: Day 0-28 after each dose vaccination
Time Frame: 28 days after the second dose vaccination

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Phase I: within 7 days after each dose vaccination
Phase II: at 30 days after the last dose vaccination and within 7 days after each dose vaccination

Time Frame: 7 days after each dose.

Time Frame: 1 months after immunization in each study group
Time Frame: 0 to 7 days after vaccination in each study group

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Not Specified

Not Specified

7 days

Not Specified

28 days

Time (within 28 days after randomization)

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Time (within 28 days after randomization)

Time

Time

Not Specifed

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Not Specifed

6 weeks

Not Specifed

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Not Specified

Not Specified

Time

Time Frame: 5 months

84 days

Time Frame: 7 days
Time Frame: 12 weeks

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from the beginning of the vaccination to 28 days after the second dose vaccination
28 days after the second dose vaccination.

Time Frame: Day 0-28 after each dose vaccination
Time Frame: The 28th day after the second dose vaccination

within 14 days after each vaccination
on day 28 after boost vaccination
on day 28 after boost vaccination

Time Frame: within 14 days post each vaccination
Time Frame: Day 28 post the second vaccination
Time Frame: Day 28 post the second vaccination

within 28 d after boosting
at 14 d after boosting

Time Frame: Within 28 days after the booster dose
Time Frame: On day 14 after the booster dose

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Index	Registration No.	Journal/Registry Platform
301	ChiCTR2000029386	Chinese Medical Journal
	ChiCTR2000029386	ChiCTR
302	ChiCTR2000029741	Trials
	ChiCTR2000029741	ChiCTR
303	ChiCTR2000029777	Trials
	ChiCTR2000029777	ChiCTR
304	ChiCTR2000029972	Trials
	ChiCTR2000029972	ChiCTR
305	ChiCTR2000029993	TMR Modern Herbal Medicine
	ChiCTR2000029993	ChiCTR
306	ChiCTR2000029994	Trials

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	ChiCTR2000029994	ChiCTR
307	ChiCTR2000030043	Trials
	ChiCTR2000030043	ChiCTR
308	ChiCTR2000030703	Trials
	ChiCTR2000030703	ChiCTR
309	ChiCTR2000030759	Trials
	ChiCTR2000030759	ChiCTR
310	NCT04336254 (ChiCTR2000031319)	Trials
	NCT04336254	ClinicalTrials.gov
311	ChiCTR2000033284	Infectious Diseases of Poverty
	ChiCTR2000033284	ChiCTR
312	ChiCTR2000037524	Infectious Diseases of Poverty

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	ChiCTR2000037524	ChiCTR
313	NCT04257656	Trials
	NCT04257656	ClinicalTrials.gov
314	NCT04264533	BMJ Open
	NCT04264533	ClinicalTrials.gov
315	NCT04333589	Trials
	NCT04333589	ClinicalTrials.gov
316	NCT04334265	Trials
	NCT04334265	ClinicalTrials.gov
317	ChiCTR2000029869	Trials
	ChiCTR2000029869	ChiCTR
318	ChiCTR2000032367	Trials

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	ChiCTR2000032367	ChiCTR
319	ChiCTR2000032573	Pulmonary Circulation
	ChiCTR2000032573	ChiCTR
320	ChiCTR2000032737	Current Medical Science
	ChiCTR2000032737	ChiCTR

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DOI	Publication Title/Registration (Official/Scientific) Title
10.1097/CM9.0000000000000791	Effectiveness of glucocorticoid therapy in patients with severe coronavirus disease 2019: protocol of a randomized controlled trial A study on treatment strategies of novel coronavirus pnueumonia patients
10.1186/s13063-020-04478-w	Efficacy of chloroquine versus lopinavir/ritonavir in mild/general COVID-19 infection: a prospective, open-label, multicenter, randomized controlled clinical Efficacy of Chloroquine and Lopinavir/ Ritonavir in mild/general novel coronavirus (CoVID-19) infections: a prospective, open-label, multicenter randomized controlled
10.1186/s13063-020-04498-6	Shenhuang granule in the treatment of severe coronavirus disease 2019 (COVID-19): study protocol for an openlabel randomized controlled clinical trial A multicenter, randomized, controlled trial for integrated chinese and western medicine in the treatment of novel coronavirus pneumonia (COVID-19) based
10.1186/s13063-020-04534-5	The curative effects of shortwave diathermy on treating Novel coronavirus (COVID-19) pneumonia: A structured summary of a study protocol for a A randomized controlled trial for the Efficacy of Ultra Short Wave Electrotherapy in the treatment of Novel Coronavirus Pneumonia (COVID-19)
10.12032/TMRmhm202003084	Efficacy and safety of the combination of Liushen capsules and Arbidol in the treatment of COVID-19: protocol for a randomized, multi-center pilot study A multicenter, randomized, open-label, parallel controlled trial for the safety and efficacy of Integrated Chinese and Western Medicine in treatment of non-
10.1186/s13063-020-04693-5	Acupressure therapy and Liu Zi Jue Qigong for pulmonary function and quality of life in patients with severe novel coronavirus pneumonia (COVID-19): a

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	Liu-Zi-Jue Qigong and Acupressure Therapy for Pulmonary Function and Quality of Life in Patient with Severe novel coronavirus pneumonia (COVID-19): A
10.1186/s13063-020-04677-5	Impact of Shenfu injection on a composite of organ dysfunction development in critically ill patients with coronavirus disease 2019 (COVID-19): A structured Shen-Fu injection in the treatment of severe novel coronavirus pneumonia (COVID-19): a multicenter, randomized, open-label, controlled trial
10.1186/s13063-020-04925-8	Safety and Efficacy of Ixekizumab and Antiviral Treatment for Patients with COVID-19: A structured summary of a study protocol for a Pilot Randomized A randomized, blinded, controlled, multicenter clinical trial to evaluate the efficacy and safety of Ixekizumab combined with conventional antiviral drugs
10.1186/s13063-020-04970-3	Efficacy of Traditional Chinese Medicine, Maxingshigan-Weijing in the management of COVID-19 patients with severe acute respiratory syndrome: A structured Study for the therapeutic effect and mechanism of traditional Chinese medicine in the treatment of novel coronavirus pneumonia (COVID-19)
10.1186/s13063-020-04380-5	Safety and efficacy assessment of allogeneic human dental pulp stem cells to treat patients with severe COVID-19: structured summary of a study protocol for Safety and Efficacy Study of Allogeneic Human Dental Pulp Mesenchymal Stem Cells to Treat Severe Pneumonia of COVID-19:a Single-center, Prospective,
10.1186/s40249-021-00813-8	Efcacy of the combination of modern medicine and traditional Chinese medicine in pulmonary fbrosis arising as a sequelae in convalescent COVID-19 patients: a Study for the improvement of the quality of life of severe patients of novel coronavirus pneumonia (COVID-19) by the integrated Chinese medicine and Western medicine
10.1186/s40249-021-00836-1	Mindfulness-based online intervention on mental health and quality of life among COVID-19 patients in China: an intervention design

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	Longitudinal research project of Wuhan novel coronavirus pneumonia (COVID-19) patients and important rehabilitation factors
10.1186/s13063-020-04352-9	Evaluation of the efficacy and safety of intravenous remdesivir in adult patients with severe COVID-19: study protocol for a phase 3 randomized, double-blind, A Phase 3 Randomized, Double-blind, Placebo-controlled, Multicenter Study to Evaluate the Efficacy and Safety of Remdesivir in Hospitalized Adult Patients
10.1136/bmjopen-2020-039519	Intravenous high-dose vitamin C for the treatment of severe COVID-19: study protocol for a multicentre randomised controlled trial Vitamin C Infusion for the Treatment of Severe 2019-nCoV Infected Pneumonia: a Prospective Randomized Clinical Trial
10.1186/s13063-020-04430-y	The Mechanism and Clinical Outcome of patients with Corona Virus Disease 2019 Whose Nucleic Acid Test has changed from negative to positive, and the The Mechanism, Clinical Outcome and Therapeutic Intervention of Corona Virus Disease 2019 Patients Whose Nucleic Acids Changed From Negative to Positive
10.1186/s13063-020-04399-8	Efficacy and safety of Anluohuaxian in the treatment of patients with severe Coronavirus disease 2019- a multicenter, open label, randomized controlled study: a Efficacy and Safety of Anluohuaxian in the Treatment of Rehabilitation Patients With Corona Virus Disease 2019-A Multicenter, Open, Randomized Controlled Study
10.1186/s13063-021-05418-y	Baidu Jieduan granules, traditional Chinese medicine, in the treatment of moderate coronavirus disease-2019 (COVID-19): study protocol for an open- A multicenter, randomized, controlled trial for integrated Chinese and western medicine in the treatment of ordinary novel coronavirus pneumonia (COVID-19) based
10.1186/s13063-020-04383-2	Efficacy of Liu-zi-jue in Patients with 2019 Novel Coronavirus Pneumonia (COVID-19): structured summary of a study protocol for a randomized controlled trial

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	Rqesearch on the Application of Traditional Chinese Medicine Traditional Techniques to Prevent and Control New Coronary Pneumonia
10.1177/20458940211032125	Bufei huoxue capsules in the management of convalescent COVID-19 infection: study protocol for a multicenter, double-blind, and randomized controlled trial A randomized, double-blind, controlled trial for Bufei Huoxue Capsule in the treatment of convalescent COVID-19 patients
10.1007/s11596-021-2475-2	Efficacy and Safety of Washed Microbiota Transplantation to Treat Patients with Mild-to-Severe COVID-19 and Suspected of Having Gut Microbiota Dysbiosis: Study Clinical trial for the washed microbiota transplantation in the treatment of novel coronavirus pneumonia (COVID-19) patients suspected with gut microbiota

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Deviation in Estimated Enrollment (1 if Yes, 0 if not specified in one or more of data sources)	Absoulute Deviation by Precentage	Estimated Enrollment	Deviation in Center (1 if Yes, 0 if not specified in one or more of data sources)
		48	
		48	
		112	1
		112	
		160	1
		160	
1	925%	410	
		40	
		40	0
		40	
1	36%	128	

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		200	
		300	1
		300	
		40	
		40	
1	100%	140	
		70	
		20	
		20	
1	242.67%	514	0
		150	
		1600	1

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		1600	
1	0.22%	453	0
		452	
1	120%	308	1
		140	
		210	1
		210	
		750	
		750	
		300	1
		300	
		186	0

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	186	
	120	0
	120	
0	#BLANK	1
	60	

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Center	Deviation in Inclusion Criteria (1 if Yes, 0 if not specified in one or more of data sources)
Chongqing Public Health Medical Center	
Chongqing Public Health Medical Center	
Fifth Affiliated Hospital of Sun Yat-sen University the Ninth People's Hospital of Dongguan Zhongshan Second People's Hospital The Fifth Affiliated Hospital Sun Yat-Sen University	1
Huangshi Hospital of Traditional Chinese Medicine Tongji Hospital Tongji Medical College Huazhong University of Science and Huangshi Hospital of traditional Chinese Medicine Tongji Hospital of Huazhong University of science and technology	1
Tongji University Hospital (Huazhong University of Science and Technology, Wuhan, China) Tongji Hospital, Tongji Medical College, Huazhong University of Science and Technology	
Not specified	
The First Affiliated Hospital of Guangzhou Medical University Xianning Central Hospital First People's Hospital of Yunnan Province	
Huangshi Hospital of Traditional Chinese Medicine	1

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Huangshi city hospital of TCM	
The First Affiliated Hospital of Bengbu Medical College Henan Provincial People's Hospital The First Affiliated Hospital of Nanchang The first Affiliated Hospital of AnHui Medical University Anhui Provincial Hospital for Infection The First Affiliated Hospital of Bengbu	1
Xiangya Hospital of Central South University the Third Xiangya Hospital of Central South University Xiangya Hospital (West District of Wuhan Union Medical College Hospital) Third Xiangya Hospital (Sino-French New Hospital District of Wuhan Tongji Hospital)	
the First Affiliated Hospital of Wenzhou Medical University the Second Affiliated Hospital of Wenzhou Medical University The First Affiliated Hospital of Wenzhou Medical University Wenzhou city center hospital medical group The second hospital affiliated to wenzhou	1
Renmin Hospital of Wuhan University	
Renmin Hospital of Wuhan University (East Campus)	
Not specified	1
Guangzhou Women and Children's Medical Center Guangzhou Eighth People's Hospital	
Hubei Provincial Hospital of Integrated Traditional Chinese and Western Medicine	1

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Wuhan Jinyintan Hospital	
Jin Yin-tan Hospital Tongji Hospital, Tongji Medical College of Huazhong University of Science & Technology	1
Not specified	
Zhongnan Hospital of Wuhan University Leishenshan Hospital of Wuhan City	1
Zhongnan Hospital of Wuhan University	
Wuhan Pulmonary Hospital, Hubei Jinyintan Hospital of Wuhan, Hubei Ezhou Central Hospital, Hubei The Second People's Hospital of Fuyang, The Second People's Hospital of Fuyang Ezhou Hospital of Traditional Chinese Medicine Ezhou Central Hospital	1
The Second People's Hospital of Fuyang Ezhou Central Hospital Huoshenshan Hospital of Wuhan Jinyintan Hospital of Wuhan The Second People's Hospital of Fuyang Ezhou Central Hospital Huoshenshan Hospital of Wuhan Jinyintan Hospital of Wuhan	
the Huangshi Hospital of Traditional Chinese Medicine Tongji Hospital, Tongji Medical College, Huazhong University of Science and Huangshi Hospital of Traditional Chinese Medicine Tongji Hospital of Huazhong University of science and technology	1
Not specified	

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Yueyang Integrated Traditional Chinese and Western Medicine Hospital Affiliated to Shanghai University of Traditional Chinese Medicine

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The First Affiliated Hospital of Guangzhou Medical University
Xiangzhou District People's Hospital
Third Hospital of Baotou City

Guangzhou Eighth People's Hospital 1

The First Affiliated Hospital of Guangdong Pharmaceutical University
Guangzhou Eighth People's Hospital

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Inclusion Criteria
(1) Aged 18 years or over; (2) Severe COVID-19; (3) Be willing to give informed consent.
1. Male or female over 18 years of age; 2. Novel coronavirus infection is confirmed by pathogenic detection; 3. The diagnosis of severe coronavirus pneumonia will have to meet at least one of the following criteria:
1. Age ≥ 18 years 2. Patients diagnosed with COVID-19 according to the criteria of the seventh editio n of the Treatment Plan of China for new coronavirus pneumonia (URL: http://202. 116.81.74/cache/4/03/www.nhc.gov.cn/5d9aa1423a8a577e1cc197a0d3c434d8/ce
1. Aged >= 18 years; 2. Meet all the following criteria (refer to confirmed cases in the 5th edition of the diagnosis and treatment plan): (1) Epidemiological history;
1. Hospitalized patients with 2019 novel coronavirus confirmed by SARS-CoV-2 nucleic acid testing of the respiratory specimens which showed positive results by means of real-time reverse transcription polymerase chain reaction (RT-PCR) assay.
1. Hospitalized patients with novel coronavirus pneumonia confirmed by pathogenic detection 2. Meet any one of the criteria for severe type: 1) Respiratory distress: RP >= 30/min;
(1) Age 18-65 years (2) COVID-19 nucleic acid test is positive (3) Lung CT showed multiple patchy ground glass shadows or other typical manifestations of both lungs
(1) Aged 18-65 years; (2) Virus nucleic acid test is positive; (3) Lung CT showed multiple patchy, lumpy ground glass opacity(GGO), consolidation or other typical manifestations in both lungs .
(1) Aged 18 years or older, male or female; (2)Fever, axillary temperature > 37.3℃; (3)Mild, common and severe patients with confirmed novel coronavirus infection according to the guideline (it will be updated accordingly if the guideline of NCP
1. Aged>= 18, male or female; 2. Fever, axillary temperature > 37.3 degrees; 3. Consistent with the clinical diagnosis of COVID-19 mild, normal, severe case; 4. Patients who sign informed consent form freely and voluntarily.
(1) meet the critical diagnosis criteria for severe COVID-19, (2) are aged between 20 and 80 years and are male or female, (3) have a stable condition and are conscious and cooperative in the examination, (4) volunteer to join the trial and sign the informed consent form, and

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(1) Meet NCP critical diagnosis criteria;

(2) Volunteer to participate in this study, cooperate with examination and treatment;

(3) The selected patients were basically stable, conscious and cooperative;

The patients, who are aged from 18 to 75 years old, with a confirmed or suspected diagnosis of severe or critical COVID-19, will be consecutively recruited in the study, according to the guideline on diagnosis and treatment of COVID-19 (the 7th version) issued by National Health Commission of the People's Republic of China.

1. Confirmed and suspected cases of pneumonia from novel coronavirus infection;

2. Meet the diagnostic criteria for community - acquired pneumonia;

3. Clinical classification is "heavy" or "critical";

4. Aged 18 to 75 years;

the age is 18-75 years old, regardless of sex;

COVID-19 will be diagnosed by RT-PCR or gene sequencing;

the clinical types are common type (including severe high-risk factors) (multiple lesions of both lungs, or lung imaging showing that the lesion progressed more

1. Aged 18 to 75 years old;

2. The diagnosis of new-type coronavirus pneumonia was confirmed by RT-PCR or gene sequencing according to the "Diagnosis and Treatment Scheme for Pneumonia of New Coronavirus Infection (Trial Version 6 and Updated)", and the

1. Participants are 18-85 years of age, either male or female.

2. Diagnosed as positive for severe acute respiratory syndrome coronavirus 2 (SARS-CoV-2)

3. Symptomatic. Mild (mild clinical symptoms without signs of pneumonia in chest

(1) Patients with Novel coronavirus pneumonia diagnosed according to <diagnosis and treatment scheme for pneumonia of new coronavirus infection (trial version 5)>;

(2) Inpatients aged >18 years old;

1) Adults aged 18-65 years;

2) Voluntarily participate in this clinical trial and sign the "informed consent form" or have consent from a legal representative.

3) Diagnosed with severe pneumonia of COVID-19: nucleic acid test SARS-CoV-2

Aged 18-65 years;

Voluntarily participate in this clinical trial and sign off "informed consent form";

Diagnosed with severe pneumonia of COVID: respiratory distress, RR >30 times / min; resting oxygen saturation of 93% or less; arterial partial pressure of oxygen /

(1) Diagnostic criteria for severe or critical COVID-19 Patients exhibiting positive et iological evidence of the SARS-CoV-2 nucleic acid determined using real-time poly merase chain reaction (PCR) or gene sequencing will be included. In addition, pati ents should meet at least one of the following diagnostic criteria of severe COVID-

1. The novel coronavirus pneumonia, once diagnosed as a severe or critical illness, has been cured and discharged without any pathogen or rehabilitation;

2. It conforms to the above diagnostic standards of Western medicine and TCM syndrome differentiation;

1) Over 18 and under 65 years old

2) Have a history of COVID-19 infection

3) Proficiency in Chinese

4) Be able to independently cooperate with doctors to complete various scale

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1. Subjects over 18 years old;
2. Patients with novel coronavirus pneumonia history;
3. Those who can read Chinese characters;
4. Those who can independently cooperate with doctors to complete various

1. Age ≥ 18 years at time of signing of informed consent form
2. Laboratory (reverse transcription polymerase chain reaction [RT-PCR])-confirmed COVID-19
3. Pneumonia confirmed with chest imaging

Age ≥18 years at time of signing Informed Consent Form

Laboratory (RT-PCR) confirmed COVID-19.

Lung involvement confirmed with chest imaging

Hospitalized with a SaO2/SPO2≤94% on room air or Pa02/Fi02 ratio <300mgHg

1. Adults (age 18 years or older).
2. Diagnosed with COVID-19 according to the Diagnosis and Clinical Management of 2019 novel coronavirus infected pneumonia (trial version 5, revised version).
3. Having a respiratory failure index (RIF) (arterial oxygen tension (or

≥ 18 years old;

Diagnosed as serious or critical SARI (according to the 4th version of Diagnosis and Clinical management of 2019-nCoV infected pneumonia);

Being treated in the ICU.

1. Adults 18 to 80 years, male or female.
2. After the first diagnosis and treatment of COVID-19, the nucleic acid test of respiratory specimens such as sputum or nasopharyngeal swabs, has been negative for two consecutive times (sampling time interval of at least 24 hours), in

COVID-19 has been diagnosed, and the nucleic acid test of respiratory specimens such as sputum or nasopharyngeal swabs has been negative for two consecutive times after treatment (sampling time interval of at least 24 hours);

The nucleic acid test of specimens such as sputum, throat swabs, blood, feces and

1. Confirmed COVID-19, and the nucleic acid test of respiratory specimens such as sputum or nasopharyngeal swabs is negative twice after the treatment (sampling interval is at least 24 hours);
2. Negative nucleic acid test of respiratory specimens such as sputum or

Confirmed COVID-19, and the nucleic acid test of respiratory specimens such as sputum or nasopharyngeal swabs is negative twice after the treatment (sampling interval is at least 24 hours);

Negative nucleic acid test of respiratory specimens such as sputum or

1. Patients 18–85 years of age
2. Patients with COVID-19 confirmed by SARS-CoV2 nucleic acid testing of respiratory specimens showing positive results
3. Patients providing written informed consent

1. patients with novel coronavirus pneumonia confirmed by pathogenic detection;
2. Fever, respiratory tract and other symptoms, the change of pneumonia can be seen in imaging;
3. Aged 18 to 80 years;

(1) There is no restrictions on gender and age;

(2) Patients diagnosed with mild pneumonia (slight clinical symptoms, no pneumonia manifestations on imaging), general pneumonia patients (with fever, respiratory tract symptoms, etc., imaging showed pneumonia but no multiple organ

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(1) There is no restrictions on gender and age;
(2) Patients diagnosed with mild pneumonia (slight clinical symptoms, no pneumonia manifestations on imaging), general pneumonia patients (with fever, respiratory tract symptoms, etc., imaging showed pneumonia but no multiple organ

1. Age 18 years old, male or female.
2. Patients who were diagnosed as COVID-19 according to the “The Diagnosis and Treatment Scheme of COVID-19 (The 7th Trial Edition)”.
3. The condition meets the discharge standards stipulated in “The Diagnosis and

1. Aged >= 18 years, male or female.
2. Patients who were diagnosed as COVID-19 according to the The Diagnosis and Treatment Scheme of COVID-19(The 7th Trial Edition).
3. The condition meets the discharge standards stipulated in the The Diagnosis

Patients diagnosed with mild-to-severe COVID-19 and suspected of having GMD will be recruited for this clinical trial.

1. COVID-19 patients accompanied with gastrointestinal symptoms;
2. COVID-19 patients accompanied with antibiotic-associated diarrhea

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Deviation in Exclusion Criteria (1 if Yes, 0 if not specified in one or more of data sources)
1
1
1
1

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1

For peer review only

1

1

For peer review only

For peer review only

Exclusion Criteria
(1) Are allergic or intolerant to any therapeutic drugs used in this study;
(2) Pregnant or lactating women;
(3) Presence of severe systemic illness that may affect the effectiveness or safety evaluation for this study.
1. Patients are allergic or intolerant to therapeutic drugs;
2. Pregnancy or lactating women;
3. Researchers believe that patients may have other factors affecting the efficacy or safety evaluation of this study.
1. Patients with a history of allergy to chloroquine phosphate, lopinavir, or ritonavir
2. Patients with hematological diseases
3. Patients with end-stage liver or kidney diseases
4. Patients with arrhythmia and/or chronic heart disease
1. Patients with a history of allergy to chloroquine phosphate, lopinavir, and ritonavir;
2. Patients with hematological diseases;
3. Patients with chronic liver and kidney disease and reaching the end stage;
4. Patients with arrhythmia and chronic heart disease;
1. Pregnant and lactating women
2. Allergic to one or more substance in “Shenhuang granule”
3. Severe primary disease including malignant tumor, blood diseases, HIV, and severe liver and kidney dysfunctions
1. Pregancy or lactating women;
2. People with allergies or allergies to components of Truncated Torsion formula;
3. Patients with serious basic diseases such as malignant tumor, cirrhosis, chronic renal failure (uremic stage), hematological disease, HIV, etc.;
(1) Patients who need ICU management,
(2) Positive tests for other pathogens such as Tuberculosis, Mycoplasma,
(3) Patients with respiratory failure or requiring mechanical ventilation,
(4) Patients with metal implants or pacemakers,
(1) Positive for other pathogens such as A, B, Mycoplasma, Adenovirus;
(2) patients require mechanical ventilation;
(3) People with metal or pacemaker;
(4) active tuberculosis;
(1) Critical patients, who fulfill one of the following conditions: I. Respiratory failure and need mechanical ventilation; II. Shock; III. Organ failure which is other than the respiratory system that requires monitor and treatment in an intensive care unit.
(2) Patients with respiratory tract infection caused by other virus except novel coronavirus;
1. Critical case with the following situation: i. Patients with respiratory failure and needing mechanical ventilation; ii. Patients with shock; iii. Patients with other organ failures and needing ICU monitoring and treatment;
2. Patients who have other respiratory tract infection;
(1) patients with complications of other serious underlying diseases, such as chronic obstructive pulmonary disease, obstructive pulmonary disease, coronary heart disease, and hypertension;
(2) patients with serious mental illness;
(3) patients with cognitive dysfunction who are unable to understand the trial process and

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1. Patients with other serious diseases such as lung diseases, cardiovascular and cerebrovascular diseases, hematopoietic diseases, autoimmune diseases, digestive system, or mental illness;
2. Pregnant or lactating women;
3. Participate in other forms of exercise during the trial

pregnant and breastfeeding women,
atopy or allergies to Shenfu Injection (SFI),
severe underlying disease (malignant tumor with multiple metastases, uncontrolled hemopathy, cachexia, severe malnutrition, HIV),

1. Pregnancy or lactating women;
2. People with allergies or allergies to Shenfu injection and its components;
3. Severe basic diseases that affect survival, including: uncontrolled malignant tumors with multiple metastases that cannot be removed, hematological diseases, cachexia, active bleeding, severe

severe new pneumonia infection with one of the following conditions: respiratory failure requiring mechanical ventilation, shock, ICU monitoring and treatment with other organ failure;
allergy to ixekizumab or antiviral drugs;
ALT/AST>5 times the ULN, neutrophils<0.5 × 109/L, or platelet less than 50 × 109/L;

1. Critical new pneumonia infection that meets one of the following conditions: respiratory failure occurs and requires mechanical ventilation; shock; combined organ failure requires ICU monitoring and treatment
2. Known allergies to Ixekizumab or antiviral drugs;

1. < 18 or > 85 years old.
2. Pregnancy and lactation.
3. Serious heart, liver, kidney and hematopoietic system diseases, abnormal liver or kidney function.

(1) Death is expected within 48 hours;
(2) Basic diseases such as primary immunodeficiency disease, acquired immunodeficiency syndrome, congenital respiratory malformation , congenital heart disease, gastroesophageal reflux disease, congenital pulmonary airway malformation and more with the exact evidence of bacterial

1. Patients have received other experimental treatment for COVID-19 within the last 30 days;
2. Patients have severe liver condition (e.g., Child Pugh score >=C or AST> 5 times of the upper limit);
3. Patients with severe renal insufficiency (estimated glomerular filtration rate <=30mL / min/1.73

Receive any clinical trial drug treatment for COVID-2019 within 30 days before the screening assessment;
Severe liver disease (e.g., Child Pugh score >=C or AST> 5 times of the upper limit);
Patients with known severe renal insufficiency (estimated glomerular filtration rate <=30mL /

First, diagnosis of any lung diseases, such as bronchiectasis, tuberculosis, bronchial asthma, chronic obstructive pulmonary disease (COPD), dyspnea, respiratory failure, lung cancer, or those with a history of these diseases.
Second, patients with a status of acute exacerbation of pulmonary fibrosis.

1. Patients with bronchiectasis, tuberculosis, asthma, chronic obstructive pulmonary disease, lung cancer and other primary lung diseases;
2. Progressive exacerbation of dyspnea symptoms in the past 6 months;
3. In the acute exacerbation stage of pulmonary fibrosis;

1) Have serious cognitive impairment.
2) Have serious heart, brain, lung, kidney, liver, and other medical diseases or tumors.
3) Difficult to cooperate with the questionnaire survey and intervention.

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1. Patients with severe mental illness or suicidal tendency;
2. Patients with serious heart, brain, lung, kidney, liver and other medical diseases or tumors;
3. Patients who can not cooperate with the investigation and intervention;
4. Pregnant or lactating women.

1. Physician makes a decision that trial involvement is not in patient's best interest, or patient has any condition that does not allow the protocol to be followed safely
2. Known severe liver disease (e.g. cirrhosis, ALT > 5 × upper limit of normal (ULN), or AST > 5 × ULN)

Physician makes a decision that trial involvement is not in patients' best interest, or any condition that does not allow the protocol to be followed safely.

Severe liver disease (e.g. Child Pugh score ≥ C, AST>5 times upper limit)

Pregnant or breastfeeding, or positive pregnancy test in a predose examination

1. Patients with a history of allergy to VC.
2. Patients with dyspnoea caused by cardiogenic pulmonary oedema.
3. Pregnant or lactating women.
4. Patients with an expected survival duration of less than 24 hours.

Allergic to vitamin C;

Dyspnea due to cardiogenic pulmonary edema;

Pregnant or breastfeeding;

Expected life is less than 24 hours;

1. Allergic to Favipiravjr;
2. Pregnant or lactating women
3. Uncontrolled diseases of the blood and cardiovascular system, liver or kidney.
4. History of mental disorders, drug abuse or dependence;

Those allergic to fapilavir;

Pregnant or lactating women;

Unstable liver, kidney, and heart diseases;

History of mental disorders, substance abuse or dependence;

1. Combined with severe heart, lung (diagnosed with interstitial lung disease, bronchial asthma, chronic obstructive pulmonary disease, etc.), liver and kidney disease or with endocrine, rheumatic, neurologic, malignant and other systemic diseases;
2. Have been diagnosed with connective tissue disease;

Combined with severe heart, lung (diagnosed with interstitial lung disease, bronchial asthma, chronic obstructive pulmonary disease, etc.), liver and kidney disease or with endocrine, rheumatic, neurologic, malignant and other systemic diseases;

Have been diagnosed with connective tissue disease;

1. Pregnancy and lactation
2. Severe primary diseases influencing survival, including malignant tumours, haemorrhagic disease and HIV
3. Severe liver and kidney dysfunction

1. Pregancy or lactating women;
2. People with allergies or allergies to traditional Chinese medicine
3. Severe Primary disease such as uncontrolled and metastasis malignancy, hematopathy, HIV;
4. Obstructive pneumonia caused by lung tumors, severe pulmonary interstitial fibrosis, alveolar

(1) patients with severe diseases such as cardiovascular, cerebrovascular, hematopoietic, digestive system or mental illness;

(2)pregnant and lactating women;

(3) respiratory frequency> 30 times /min, showing respiratory failure;

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(1) patients with severe diseases such as cardiovascular, cerebrovascular, hematopoietic, digestive system or mental illness;
(2) pregnant and lactating women;
(3) respiratory frequency> 30 times /min, showing respiratory failure;

1. Known or suspected allergy to the components of BFHX.
2. Acute infections, along with other viruses or bacteria.
3. Abnormal liver and kidney function tests (ALT, AST, SCr 1.5 times the upper limit of normal).
4. Participating in other drug clinical trials.

1. Known or suspected allergy to the components of BFHX.
2. Acute infections, along with other viruses or bacteria.
3. Abnormal liver and kidney function tests (ALT, AST, SCr >= 1.5 times the upper limit of normal).
4. Participating in other drug clinical trials.

critical type of COVID-19 according to the COVID-19 diagnosis and treatment guidelines in China (7th edition),
gastrointestinal perforation,
gastrointestinal obstruction.

(1) Critical type of COVID-19 according to COVID-19 diagnosis and treatment guidelines in China (7th edition);
(2) Gastrointestinal perforation;
(3) Gastrointestinal obstruction;

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Deviation in Masking (1 if Yes, 0 if not specified in one or more of data sources)	Masking	Deviation in Primary outcome (1 if Yes, 0 if not specified in one or more of data
0	Open	
	Not stated	
	Open	1
	Open	
0	Open	1
	Not stated	
0	Open	1
	Not stated	
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	Open	
	Open	1

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	Double	
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	Open	
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0	Open	1
	Not stated	
	Open	

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	Open	
	Double	1
	Double	
0	Single	1
	Not stated	

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Primary Outcome
the change in sequential organ failure assessment (SOFA) at 3 days after randomization SOFA score
the clinical recovery time of no more than 28 days after the completion of therapy and follow-up Length of stay Length of severe oxygenation index during treatment all-cause mortality in 28 days
clinical improvement up to 14 days after treatment 14 day outcome of the subjects, including: recovery, improvement, turning critical, death. lung CT
time to positive-to-negative conversion of COVID-19 nucleic acid test by pharyngeal swab, in days assessed at 7th, 14th ,21st and 28th days the rate of Coronary virus nucleic acid negative at 7 days, 14 days, 21 days, and 28 days after Ultra Short Wave Electrotherapy systemic inflammatory response syndrome (SIRS) scores at 7 days, 14 days, 21 days, and 28 days after Ultra Short Wave Electrotherapy
Time to major symptomatic remission: The patient's axillary temperature remains normal for >24 hours (without antipyretic drugs or corticosteroids), and the patient feels no dyspnea (for patients without dyspnea) or relief of dyspnea, oxygen saturation (SpO2) ≥ Main symptom relief time
Modified Medical Research Council (mMRC) Dyspnea Scale

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lung function
ADL
6min walk

The primary endpoint is a composite of newly developed or exacerbated organ dysfunction. This is defined as an increase in the sequential organ failure assessment (SOFA) score of two or more, indicating sepsis and involvement of at least one organ. The SOFA

pneumonia severity index (PSI)
Incidence of new organ dysfunction

change in pulmonary CT severity score (an imaging tool for assessing COVID-19, which scores on the basis of all abnormal areas involved) assessed on the 7th day, 14th day, or at discharge

Lung CT

the number of days until the clinical symptom of fever improves in the first 14 days of treatment following randomisation

The time for the positive nucleic acid of the novel coronavirus to turn negative
Incidence of deterioration
The defervescence time

Time To Clinical Improvement (TTCI)

TTCI

The main outcome measure is the treatment effect, which will measure the classification indicators, such as improvement, stabilization, and deterioration levels.
Vital Capacity
Total Vital Capacity
Forced Vital Capacity
The Predicted Forced Vital Capacity
PTSD, depression, anxiety and peace of mind
The psychological impact of COVID-19 will be assessed by the 22-item Impact of Event Scale-Revised (IES-R). IES-R is composed of three subscales and aims to measure the mean avoidance, intrusion,

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Psychological status
Sleep status

time to clinical improvement (TTCI) (censored at day 28)

Time to Clinical Improvement (TTCI)

ventilator-free days in the 28 days since admission to the ICU

Ventilation-free days

Viral nucleic acid test negative [Time Frame: 5 months]: Subjects who tested negative for nucleic acid from sputum or nasopharyngeal swabs for two consecutive times (sampling time interval of at least 24 hours).

Viral nucleic acid test negative conversion rate

1. Changes in high-resolution computer tomography of the lung
Changes in ground-glass shadows, interstitial or air nodules found on high-resolution computer tomography [Time Frame: 3 months]
2. Change in 6-minute walking distance [Time Frame: 3 months]

Changes in high-resolution computer tomography of the lung
Change in 6-minute walking distance

the proportion of negative SARS-CoV-2 nucleic acid results

14 day outcome of the subjects, including: recovery, improvement, turning critical, death.
lung CT

Length of Hospital Stay
Modify Borg Dyspnea Score

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Hospital stay
Modify Borg Dyspnea Score

CT improvement rate of lung
FAI
6MWD

St George's respiratory questionnaire score
Fatigue assessment scale
Six minute walking experiment

clinical efficacy of WMT on COVID-19 patients suspected of having GMD. (reflected by the SARS-Cov-2 infection status and the recovery of the disease, which will be defined as “improvement” (from the severe type to the moderate or mild type, or from the

SARS-Cov-2 nucleic acid test

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Secondary Outcome
proportion of mechanical ventilation use at 2 and 4 weeks, mortality at 2 and 4 weeks, and duration of hospitalization
Duration of hospitalization Proportion of mechanical ventilation use Mortality
the rate of treatment success after the completion of therapy and follow-up, the time of treatment success after no more than 28 days, the rate of serious adverse events (SAEs) during the completion of
Others (liver and kidney function, muscle enzymes, myoglobin)
mortality and rate of receiving invasive ventilator in the severe category, the improvement of oxygen saturation, and Acute Physiology and Chronic Health Evaluation (APACHE-II) score
vital signs oxygenation index Routine Blood inflammatory biomarkers
nucleic acid test rate and recovery from symptoms , vital signs assessment, treatment adverse effects, Vital capacity,
time to clinical recovey the seven-category?ordinal scores at 7 days, 14 days, 21 days, and 28 days after Ultra Short Wave Electrotherapy
(1) The time when novel coronavirus nucleic acid of the respiratory tract specimens (such as throat swabs) becomes negative for two consecutive times: Record the test results of the subject's novel coronavirus nucleic acid, and when two consecutive tests are negative
Time to COVID-19 RNA convert negative Other symptoms relief rate & time Time & Rate of inflammatory biomarkers convert normal Time to recovery
Activities of Daily Living Barthel Index (ADL-BI) Scale Patient Health Questionnaire-9 (PHQ-9) Scale Respiratory Symptoms (RS) Scale

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PHQ-9
Length of hospital stay
Respiratory symptoms
immune cells
SOFA score in total
Pneumonia severity index score
Dosage of vasoactive drugs
Ventilation free days within 28 days
28-day prognosis
SOFA
Mechanical ventilation time
Length of stay in ICU
1) Pulmonary function prognostic indicators: percentage of subjects reporting each severity level (on a 7-point scale) (https://clinicaltrials.gov/ct2/show/NCT04280705). The scale is the assessment of clinical status at the first evaluation on a specified study

Lung function
Arterial blood gas analysis

TCM Syndrome Scoring System
Coronavirus nucleic acid test
Improvements of chest X-ray (CRX)
Rate of symptom recovery
Haematological
C-reactive protein
Blood gas analysis
Urine test
1. vital signs: heart rate, blood pressure (systolic blood pressure, diastolic blood pressure). During the screening period, hospitalization every day (additional time points of D1, D4, D7 30min before injection, 2h ± 30min,
Lung lesion
Immune function
Time of SARS-CoV-2 clearance
Blood test
(1) Indices reflecting respiratory function
(2) Six-minute walking distance and Borg dyspnea score
(3) Quality of life assessment indices
(4) Evaluation standard of treatment effect in TCM syndrome

#BLANK

fatigue and sleep quality
Fatigue scale-14 (FS-14) assesses the patient's level of physical fatigue and mental fatigue (Cronbach's α = 0.81) [33, 34]. The 19-item Pittsburgh Sleep Quality Index (PSQI) assesses respondents' sleep

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#BLANK

1. Percentage of subjects in each clinical category on the six-category ordinal scale (time frame: days 7, 14, 21, and 28)
2. Time to disease improvement, defined as the time to hospital discharge, or NEWS2 (National Early Warning Score 2) of ≤ 2

Clinical status

Time to Hospital Discharge OR NEWS2 (National Early Warning Score 2) of ≤ 2 maintained for 24 hours.

All cause mortality

1. ICU mortality, 28-day mortality.
2. Changes in SOFA scores.
3. Changes in plasma biomarkers of inflammation: including white cell counts, neutrophil counts/percent- age, lymphocyte counts/percentage,

28-days mortality

ICU length of stay

Demand for first aid measuments

Vasopressor days

Clinical cure [Time Frame: 5 months]:

1. Body temperature returned to normal for more than 3 days;
2. Lung image improved.
3. Clinical manifestation improved;

Clinical cure rate

Not Stated

Changes in compound physiological index

Changes in the scores of the St. George's Hospital Respiratory Questionnaire

Changes in modified British Medical Research Council Dyspnea Scale

the time to symptom recovery

the proportion of cases with reversion on chest CT

the time to negative SARS-CoV-2 nucleic acid results

the proportion of participants regrouped as severe cases

Pneumonia symptoms

oxygenation index

Routine Blood

major organ function

Fatigue Scale-14

Patient Health Questionnaire

vital signs

respiratory symptoms

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Fatigue scale-14
Patient Health Questionnaire
Clinical Respiratory Symptom Scale
Vital signs
(1) The evaluation results and changes of the St. George's Respiratory Questionnaire (SGRQ) score.
(2) According to the "Chinese Medicine Treatment" part of The Diagnosis and Treatment Plan (The 7th Trial Edition) and the

#BLANK

1) advantages of WMT over routine treatment, as reflected by the recovery time (i.e., the time from positivity to negativity by the nasopharyngeal swab test; the clearance time of SARS-Cov-2 from feces, defined as the time from positivity to negativity by the feces
Fasting blood glucose
Liver function
Renal function
Electrolyte

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New primary outcome introduced in publiucation (1 = Yes, 0 = No)	Original primary outcome omitted in publication (1 = Yes, 0 = No)
1	1
0	1
1	0
1	1

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1	1
0	1
1	1
0	1

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For peer review only

1	0

1	0

For peer review only

Secondary outcome in registry described as primary outcome in publication (1 = Yes, 0 = No)	Primary outcome in registry described as secondary outcome in publication (1 = Yes, 0 = No)
0	0
0	0
0	1
0	1

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0	1

0	1

0	1

0	1

For peer review only



For peer review only

0	1

0	1

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Deviation in time of Primary outcome Measurement (1 if Yes, 0 if not specified in one or more of data sources)	Time of Primary Outcome Measurement
0	3 days after randomization
	Not Specified
	Time
	Time

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1		on the 7th day, 14th day, or at discharge
		Days 7 and 14
		Time Frame: 1-28 days
		Time Frame: 1-28 days

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	Time (censored at day 28)
	[Censored at Day 28] Time Frame: up to 28 days
	in the 28 days since admission to the ICU
	Time Frame: on the day 28 after enrollment
	Time Frame: 5 months
	Time Frame: 5 months
	Time Frame: 3 months Time Frame: 3 months
	Time Frame: 3 months Time Frame: 3 months
0	the baseline, before treatment on the sixth day of hospitalization and the discharge day

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Not Specified

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Index	Registration No.	Journal/Registry Platform	DOI
Protocol-313	NCT04257656	Trials	10.1186/s13063-020-04352-9
Report-128	NCT04257656	Lancet	10.1016/S0140-6736(20)31022-9
Protocol-314	NCT04264533	BMJ Open	10.1136/bmjopen-2020-039519
Report-130	NCT04264533	Ann Intensive Care.	10.1186/s13613-020-00792-3
Protocol-303	ChiCTR2000029777	Trials	10.1186/s13063-020-04498-6
Report-145	ChiCTR2000029777	Phytomedicine	10.1016/j.phymed.2021.153612
Protocol-306	ChiCTR2000029994	Trials	10.1186/s13063-020-04693-5
Report-149	ChiCTR2000029994	Integrative Medicine Research	10.1016/j.imr.2021.100796
Protocol-309	ChiCTR2000030759	Trials	10.1186/s13063-020-04970-3
Report-155	ChiCTR2000030759	Integrative Medicine Research	10.1016/j.imr.2021.100782
Protocol-315	NCT04333589	Trials	10.1186/s13063-020-04430-y

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Report-164	NCT04333589	International Immunopharmacology	10.1016/j.intimp.2021.107702
Protocol-319	ChiCTR2000032573	Pulmonary Circulation	10.1177/20458940211032125
Report-157	ChiCTR2000032573	Journal of Ethnopharmacology	10.1016/j.jep.2021.114830

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Publication Title/Registration (Official/Scientific) Title
Evaluation of the efficacy and safety of intravenous remdesivir in adult patients with severe COVID-19: study protocol for a phase 3 randomized, double-blind, placebo-controlled, multicentre trial
Remdesivir in adults with severe COVID-19: a randomised, double-blind, placebo-controlled, multicentre trial
Intravenous high-dose vitamin C for the treatment of severe COVID-19: study protocol for a multicentre randomised controlled trial
Pilot trial of high-dose vitamin C in critically ill COVID-19 patients
Shenhuang granule in the treatment of severe coronavirus disease 2019 (COVID-19): study protocol for an openlabel randomized controlled clinical trial
Traditional Chinese medicine shenhuang granule in patients with severe/critical COVID-19: A randomized controlled multicenter trial
Acupressure therapy and Liu Zi Jue Qigong for pulmonary function and quality of life in patients with severe novel coronavirus pneumonia (COVID-19): a study protocol for a randomized controlled trial
Effect of qigong exercise and acupressure rehabilitation program on pulmonary function and respiratory symptoms in patients hospitalized with severe COVID-19: a randomized controlled trial
Efficacy of Traditional Chinese Medicine, Maxingshigan-Weijing in the management of COVID-19 patients with severe acute respiratory syndrome: A structured summary of a study protocol for a randomized controlled trial
Therapeutic effects of traditional Chinese medicine (Maxingshigan-Weijing Decoction) on COVID-19: An open-label randomized controlled trial
The Mechanism and Clinical Outcome of patients with Corona Virus Disease 2019 Whose Nucleic Acid Test has changed from negative to positive, and the therapeutic efficacy of Favipiravir: A structured summary of a study protocol for a randomised controlled trial

Favipiravir in the treatment of patients with SARS-CoV-2 RNA recurrent positive after discharge: A multicenter, open-label, randomized trial

Bufei huoxue capsules in the management of convalescent COVID-19 infection: study protocol for a multicenter, double-blind, and randomized controlled trial

Efficacy and safety of Bufei Huoxue capsules in the management of convalescent patients with COVID-19 infection: A multicentre, double-blind, and randomised controlled trial

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Deviation in Estimated Enrollment (1 if Yes, 0 if not specified in one or more of data sources)	Absoulute Deviation by Precentage	Actual Enrollment (Participants Randomized)/Original Estimated Enrollment in Registry
		453
1	47.70%	237
		308
1	82%	56
		160
1	23.80%	122
		128
		128
		140
1	57.90%	59
		210

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1	73.80%	55
		120
1	9.20%	131

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Deviation in Center (1 if Yes, 0 if not specified in one or more of data sources)	Center
	Jin Yin-tan Hospital Tongji Hospital, Tongji Medical College of Huazhong University of Science & Technology Wuhan Lung Hospital
0	Not specified
	Zhongnan Hospital of Wuhan University Leishenshan Hospital of Wuhan City
	Zhongnan Hospital of Wuhan University, Leishenshan (Thunder God Mountain) Hospital, Taihe Hospital
1	Huangshi Hospital of Traditional Chinese Medicine Tongji Hospital Tongji Medical College Huazhong University of Science and Technology LaoHeKou Tradiational Chinese Medicine Hospital Leishenshan Hospital of Wuhan Tongji Hospital Wuhan Mental Health Center Huangshi Hospital of TCM (The Infectious Disease
	Huangshi Hospital of Traditional Chinese Medicine
	Huangshi Hospital of Traditional Chinese Medicine
1	the First Affiliated Hospital of Wenzhou Medical University the Second Affiliated Hospital of Wenzhou Medical University First Affiliated Hospital of Wenzhou Medical University
	Wuhan Pulmonary Hospital, Hubei Jinyintan Hospital of Wuhan, Hubei Ezhou Central Hospital, Hubei The Second People's Hospital of Fuyang, Anhui

1	Wuhan Pulmonary Hospital Ezhou Central Hospital The Third People's Hospital of Shenzhen The First Affiliated Hospital of Bengbu Medical
	Not Specified
0	Not Specified

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Deviation in Inclusion Criteria (1 if Yes, 0 if not specified in one or more of data sources)
1
1
1

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1

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Inclusion Criteria

1. Age ≥ 18 years at time of signing of informed consent form
2. Laboratory (reverse transcription polymerase chain reaction [RT-PCR])-confirmed COVID-19
3. Pneumonia confirmed with chest imaging

Eligible patients were men and non-pregnant women with COVID-19 who were aged at least 18 years and were RT-PCR positive for SARS-CoV-2, had pneumonia confirmed by chest imaging, had oxygen saturation of 94% or lower on room air or a ratio of arterial oxygen partial pressure to

1. Adults (age 18 years or older).
2. Diagnosed with COVID-19 according to the Diagnosis and Clinical Management of 2019 novel coronavirus infected pneumonia (trial version 5, revised version).

(1) age ≥ 18 and < 80 years;
(2) RT-PCR positive for SARS-CoV-2;
(3) pneumonia confirmed by chest imaging and admission to the ICU;
(3) PaO2/FiO2(P/F) < 300 mmHg.

1. Hospitalized patients with 2019 novel coronavirus confirmed by SARS-CoV-2 nucleic acid testing of the respiratory specimens which showed positive results by means of real-time reverse transcription polymerase chain reaction (RT-PCR) assay.

Hospitalized patients with COVID-19 were confirmed by pathogenic detection viral diagnostic techniques (nucleic acid test).

Meet any one of the criteria in the severe or critical category according to the national guideline of diagnosis and treatment

(1) meet the critical diagnosis criteria for severe COVID-19,
(2) are aged between 20 and 80 years and are male or female,
(3) have a stable condition and are conscious and cooperative in the examination,

(1) critical diagnostic criteria for severe COVID-19
(2) aged between 20 and 80 years
(3) stable condition and conscious and cooperative during the examination
(4) willing to participate in the trial and sign the informed consent form

1. Participants are 18-85 years of age, either male or female.
2. Diagnosed as positive for severe acute respiratory syndrome coronavirus 2 (SARS-CoV-2)
3. Symptomatic. Mild (mild clinical symptoms without signs of pneumonia in

1. Age of 18-85 years, either male or female;
2. Diagnosed as positive for severe acute respiratory syndrome coronavirus 2 (SARS-CoV-2);
3. Symptomatic. Mild (mild clinical symptoms without signs of pneumonia

1. Adults 18 to 80 years, male or female.
2. After the first diagnosis and treatment of COVID-19, the nucleic acid test of respiratory specimens such as sputum or nasopharyngeal swabs, has been negative for two consecutive times (sampling time interval of at least

① Over 18 years of age, both male and female;

② After the first diagnosis and treatment of COVID-19, the SARS-CoV-2 RNA test of respiratory specimens such as sputum or nasopharyngeal swabs, has been negative for two consecutive times (sampling time interval

1. Age 18 years old, male or female.

2. Patients who were diagnosed as COVID-19 according to the “The Diagnosis and Treatment Scheme of COVID-19 (The 7th Trial Edition)”.

3. The condition meets the discharge standards stipulated in “The hospitalised patients who met the following criteria:

(1)age ≥18 years;

(2) diagnosis of COVID-19 in accordance with “The Diagnosis and Treatment Scheme of COVID-19 (7th Trial Version)”;

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Deviation in Exclusion Criteria (1 if Yes, 0 if not specified in one or more of data sources)
1
1

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Exclusion Criteria	Deviation in Masking (1 if Yes, 0 if not specified in one or more of data sources)
1. Physician makes a decision that trial involvement is not in patient's best interest, or patient has any condition that does not allow the protocol to be followed safely 2. Known severe liver disease (e.g. cirrhosis, ALT > 5 × upper	
Exclusion criteria included pregnancy or breast feeding; hepatic cirrhosis; alanine aminotransferase or aspartate aminotransferase more than five times the upper limit of normal; known severe renal impairment (estimated glomerular filtration	
1. Patients with a history of allergy to VC. 2. Patients with dyspnoea caused by cardiogenic pulmonary oedema. 3. Pregnant or lactating women.	
Exclusion criteria were allergy to vitamin C, pregnancy or breastfeeding, expected survival duration < 24 h, and previous history of glucose-6-phosphate dehydrogenase deficiency or end-stage pulmonary disease. Patients who were already enrolled in	1
1. Pregnant and lactating women 2. Allergic to one or more substance in "Shenhuang granule" 3. Severe primary disease including malignant tumor, blood diseases, HIV, and severe liver and kidney dysfunctions	
1 Pregnant or lactating women 2 Allergy to herbs of Shenhuang Granule 3 History of a severe primary disease, including unresectable tumors, blood diseases (such as acute leukemia and purpura	
(1) patients with complications of other serious underlying diseases, such as chronic obstructive pulmonary disease, obstructive pulmonary disease, coronary heart disease, and hypertension;	
(1) complications due to other underlying severe diseases, such as chronic obstructive pulmonary disease, obstructive pulmonary disease, coronary heart disease, and hypertension, (2) serious mental illness,	0
1. < 18 or > 85 years old. 2. Pregnancy and lactation. 3. Serious heart, liver, kidney and hematopoietic system diseases, abnormal liver or kidney function.	
1. < 18 or > 85 years old; 2. Pregnancy and lactation; 3. Serious heart, liver, kidney and hematopoietic system diseases, abnormal liver or kidney function;	
1. Allergic to Favipiravjr; 2. Pregnant or lactating women 3. Uncontrolled diseases of the blood and cardiovascular system, liver or kidney.	

① Allergic to favipiravir;
② Pregnant or lactating women;
③The researchers considered the patient was not suitable to participate in this clinical trial.

1. Known or suspected allergy to the components of BFHX.
2. Acute infections, along with other viruses or bacteria.
3. Abnormal liver and kidney function tests (ALT, AST, SCr 1.5 times the upper limit of normal).

(1) known or suspected allergy to the components of BFHX;
(2) acute infections caused by other viruses or bacteria;
(3) abnormal liver and kidney function tests (alanine aminotransferase [ALT], aspartate aminotransferase [AST], and

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Masking	Deviation in Primary outcome (1 if Yes, 0 if not specified in one or more of data sources)
Double	
Double	
Single	
Double	1
Open	
Open	
Open	
Not specified	
Open	
Open	1
Open	

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Open

Double

Double 1

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Primary Outcome

time to clinical improvement (TTCI) (censored at day 28)

time to clinical improvement within 28 days after randomisation.

ventilator-free days in the 28 days since admission to the ICU

invasive mechanical ventilation (IMV)-free days in 28 days (IMVFD28).

clinical improvement up to 14 days after treatment

clinical improvement

Modified Medical Research Council (mMRC) Dyspnea Scale

modified Medical Research Council (mMRC) dyspnea scale

the number of days until the clinical symptom of fever improves in the first 14 days of treatment following randomisation

the rate of symptom recovery after 14 days of treatment

Viral nucleic acid test negative [Time Frame: 5 months]: Subjects who tested negative for nucleic acid from sputum or nasopharyngeal swabs for two consecutive times (sampling

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time to achieve a consecutive twice (at intervals of more than 24 h) negative RT-PCR result for SARS-CoV-2 RNA in nasopharyngeal swab and sputum sample.

CT improvement rate of lung

FAI

6MWD

improvements in 6MWD and findings on chest computed tomography (CT) images.

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Secondary Outcome

1. Percentage of subjects in each clinical category on the six-category ordinal scale (time frame: days 7, 14, 21, and 28)
2. Time to disease improvement, defined as the time to hospital discharge, or NEWS2 (National Early Warning Score 2) of ≤ 2 maintained
the proportions of patients in each category of the six-point scale at day 7, 14, and 28 after randomisation;
all-cause mortality at day 28;
frequency of invasive mechanical ventilation;

1. ICU mortality, 28-day mortality.
2. Changes in SOFA scores.
3. Changes in plasma biomarkers of inflammation: including white cell counts, neutrophil counts/percent- age, lymphocyte counts/percentage,
28-day mortality,
organ functions and inflammatory parameters, including white blood cell counts, neutrophil counts, lymphocyte counts, procalcitonin, interleukin-6 (IL-6), and C-reactive protein (CRP).

mortality and rate of receiving invasive ventilator in the severe category,
the improvement of oxygen saturation, and
Acute Physiology and Chronic Health Evaluation (APACHE-II) score

mortality
rate of advancing to critical category
receiving an invasive ventilator in the severe category

Activities of Daily Living Barthel Index (ADL-BI) Scale
Patient Health Questionnaire-9 (PHQ-9) Scale
Respiratory Symptoms (RS) Scale
the modified Borg dyspnea scale (MBS)
fatigue Scale-14 (FS-14)
patient health questionnaire-9 scale (PHQ-9)
duration of respiratory symptoms

TCM Syndrome Scoring System
Coronavirus nucleic acid test
Improvements of chest X-ray (CRX)
Rate of symptom recovery
the time to recovery of individual symptoms,
TCM syndrome scores,
the time of conversion of SARS-CoV-2 RNA assay,
hospitalization days,

Clinical cure [Time Frame: 5 months]:
1. Body temperature returned to normal for more than 3 days;
2. Lung image improved.
3. Clinical manifestation improved;

the changes of blood routine and CRP (C-reactive protein);
the count and proportion of T lymphocyte subsets in peripheral blood and the changes of cytokines

(1) The evaluation results and changes of the St. George's Respiratory Questionnaire (SGRQ) score.
(2) According to the "Chinese Medicine Treatment" part of The Diagnosis and Treatment Plan (The 7th Trial Edition) and the "Recommendation on Chinese medicine syndrome scores,
St. George's Respiratory Questionnaire (SGRQ) score,
Borg-Dyspnea Scale scores,
Fatigue Assessment Inventory (FAI) scores

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New primary outcome introduced in report (1 = Yes, 0 = No)	Original primary outcome omitted in report (1 = Yes, 0 = No)
1	1
0	0

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0 0

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Secondary outcome in protocol described as primary outcome in report (1 = Yes, 0 = No)	Primary outcome in protocol described as secondary outcome in report (1 = Yes, 0 = No)
0	0
1	1

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0 1

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Deviation in time of Primary outcome Measurement (1 if Yes, 0 if not specified in one or more of data sources)	Time of Primary Outcome Measurement
	Time (censored at day 28)
	Time
	14 days
1	
	up to 75 days
	The assessments of the clinical scales will be performed at three points (before treatment, the 7th day during hospitalization, and
1	The outcomes were evaluated at two points (pretreatment and discharge) to reflect the changes in dyspnea and clinical

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Index	Registration No.
901-1	ChiCTR2000029606 (Case Series)
901-2	ChiCTR2000029606-Report2 (Non-randomized)
902	ChiCTR2000029978 (Retrospective)
903	ChiCTR2000030627 (Case Series)
904	ChiCTR2000033320 (Retrospective Cohort)
905	ChiCTR2000034794 (Observational study)
906	ChiCTR2000029601-Report3 (Retrospective Cohort)
911	ChiCTR2000031955 (Cross-sectional)

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Journal/Registry Platform	DOI
Front. Med.	10.1007/s11684-020-0810-9
Clinical and Translational Medicine	10.1002/ctm2.297
上海中医药杂志	10.16305/j.1007-1334.2020.05.095
Chin Crit Care Med，	10.3760/cma.j.cn121430-20200810-00568
Journal of Integrative Medicine	10.1016/j.joim.2020.10.002
Inflammation Research	10.1007/s00011-020-01429-8
Pharmacological Research	10.1016/j.phrs.2020.105127
Front. Public Health,	10.3389/fpubh.2021.722604

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Publication Title/Registration (Official/Scientific) Title

Clinical study using mesenchymal stem cells for the treatment of patients with severe COVID-19

Evaluation of the safety and efficacy of using human menstrual blood-derived mesenchymal stromal cells in treating severe and critically ill COVID-19 patients: An exploratory clinical trial

308 例新型冠状病毒肺炎病例回顾性分析及抗疫强身功运动处方的临床应用方案

恢复期血浆治疗在新型冠状病毒肺炎中的应用效果分析

Effects of Tanreqing Capsule on the negative conversion time of nucleic acid in patients with COVID-19: A retrospective cohort study

Relationship between the dynamic changes of serum 2019-nCoV IgM/IgG and patient immunity after 6 month hospital discharge

Hanshiyi Formula, a medicine for Sars-CoV2 infection in China, reduced the proportion of mild and moderate COVID-19 patients turning to severe status: A cohort study

Effect of Knowledge/Practice of COVID-19 Prevention Measures on Return-to-Work Concerns; Attitudes About the Efficacy of Traditional Chinese Medicine: Survey on Supermarket Staff in Huanggang, China

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PRISMA 2020 for Abstracts Checklist

Section and Topic	Item #	Checklist item	Reported (Yes/No)
TITLE			
Title	1	Identify the report as a systematic review.	Yes
BACKGROUND			
Objectives	2	Provide an explicit statement of the main objective(s) or question(s) the review addresses.	Yes
METHODS			
Eligibility criteria	3	Specify the inclusion and exclusion criteria for the review.	Yes
Information sources	4	Specify the information sources (e.g. databases, registers) used to identify studies and the date when each was last searched.	Yes
Risk of bias	5	Specify the methods used to assess risk of bias in the included studies.	Yes
Synthesis of results	6	Specify the methods used to present and synthesise results.	Yes
RESULTS			
Included studies	7	Give the total number of included studies and participants and summarise relevant characteristics of studies.	Yes
Synthesis of results	8	Present results for main outcomes, preferably indicating the number of included studies and participants for each. If meta-analysis was done, report the summary estimate and confidence/credible interval. If comparing groups, indicate the direction of the effect (i.e. which group is favoured).	Yes
DISCUSSION			
Limitations of evidence	9	Provide a brief summary of the limitations of the evidence included in the review (e.g. study risk of bias, inconsistency and imprecision).	No, because this is covered in “Strengths and limitations of this study” section.
Interpretation	10	Provide a general interpretation of the results and important implications.	Yes
OTHER			
Funding	11	Specify the primary source of funding for the review.	No, the review was not funded.
Registration	12	Provide the register name and registration number.	No, the review was not registered.

From: Page MJ, McKenzie JE, Bossuyt PM, Boutron I, Hoffmann TC, Mulrow CD, et al. The PRISMA 2020 statement: an updated guideline for reporting systematic reviews. BMJ 2021;372:n71. doi: 10.1136/bmj.n71

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PRISMA 2020 for Abstracts Checklist

For more information, visit: http://www.prisma-statement.org/

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BMJ Open

From registration, protocol to report: are COVID-19 related RCTs in mainland China consistent? A systematic review of clinical trial registry and literature

Journal:	*BMJ Open*
Manuscript ID	bmjopen-2021-058070.R2
Article Type:	Original research
Date Submitted by the Author:	01-Jul-2022
Complete List of Authors:	Chen, Yu; The Chinese University of Hong Kong Yan, Ruiqing; Fudan University
Primary Subject Heading:	Evidence based practice
Secondary Subject Heading:	Medical publishing and peer review, Research methods, Infectious diseases
Keywords:	COVID-19, Clinical trials < THERAPEUTICS, STATISTICS & RESEARCH METHODS

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Title

From registration, protocol to report: are COVID-19 related RCTs in mainland China consistent? A systematic review of clinical trial registry and literature

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Abstract

Objective

To provide a comprehensive review of registered COVID-19 related RCTs in mainland China and evaluate the transparency of reporting through comparison of registrations, protocols and full reports.

Design

Systematic review of trial registrations and publications.

Data Sources

International Clinical Trials Registry Platform, Chinese Clinical Trial Registry, ClinicalTrials.gov, the ISRCTN registry, and EU Clinical Trial Register were accessed on 1-Feb-2022. Publications were searched in PubMed, Embase, Cochrane Library, Google Scholar, CNKI.net and Wanfangdata from 10-Feb-2022 to 12-Feb-2022.

Eligibility Criteria

Eligible trials were COVID-19 related RCTs carried out in mainland China. Observational studies, non-randomized trials and single-arm trials were excluded.

Data Extraction and Synthesis

Two reviewers independently extracted data from registrations, publications and performed risk of bias assessment for trial reports. Information provided by registrations and publications was compared. The findings were summarized with descriptive statistics.

Results

The number of eligible studies was 415. From these studies 20 protocols and 77

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RCT reports were published. Seven trials published both protocol and RCT full report. Between registrations and publications, discrepancy was found in sample size (9, 39.1% for protocols and 54, 60.0% for reports, same below), trial setting (16, 69.6% and 49, 54.4%), inclusion criteria (15, 65.2% and 67, 74.4%), exclusion criteria (12, 52.2% and 65, 72,2%), masking method (10, 43.5% and 44, 48.9%) and primary outcome or time frame of primary outcome measurement (17, 73.9% and 59, 65.6%). Between protocols and full reports, 5 (71.4%) reports had discrepancy in primary outcome or time frame of primary outcome measurement.

Conclusions

Discrepancy among registrations, protocols and reports revealed compromised transparency in reporting of COVID-19 related RCTs in mainland China. The importance of trial registration should be further emphasized to enhance transparent RCT reporting.

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Strengths and limitations of this study

- The study provided a full coverage of publicly registered RCTs related to the prevention, treatment or prognosis of COVID-19 infection.
- The study identified publications citing these registration records and examined the consistency of methodology among registrations, protocols and full reports.
- The study included only RCTs performed in mainland China.
- RCT reports that did not cite any registration number (if any) were not included in the analysis.

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Introduction

Evidence-based medicine aims to achieve optimal decision making in the care of individual patients, using the current best evidence[1]. Among all types of evidence, randomized controlled trials (RCT), together with systematic reviews of RCTs, are accorded the highest level of credibility and thus play an important role in evidence-based medicine[2]. However, not all RCTs are of the same quality, which leads to clinical research methodologists' emphasis on transparency in RCT reporting[3].

"The whole of medicine depends on the transparent reporting of clinical trials."[4]

Transparent reporting requires a complete description of methodology through which the trial data are collected and analyzed, a report without omitting any data generated by the trial and a standard way of writing[3]. Much effort has been spent on promoting transparent reporting, including the International Committee of Medical Journal Editors (ICMJE) member's requirement for registration in public trial registry prior to enrollment[5] and the announcement of CONSORT statement[6]. However, the overall transparency of RCTs remains suboptimal, and incomplete or selective reporting in publication remains an issue. Before mandatory clinical trial registration was enforced by ICMJE in 2005, Chen *et al.* reported 62% published trial report had modified, introduced or omitted in comparing the trial protocols approved by the Scientific-Ethical Committees for Copenhagen and Frederiksberg[7]; when similar approach was applied to trials approved for funding by the Canadian Institutes of Health Research, researchers found that primary outcome differed between reports and protocols in 40% of the trials [8].

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To ensure transparency in trial reporting and reduce selective reporting, the ICMJE initiative encouraged researchers to make trial information available to the public[5]. Since then, the number of registrations has increased greatly[9,10]. Prospective registration is a powerful tool in reducing selective reporting as it reflects researchers' intention at planning stage of the trials, and can be compared with published full reports[11]. Mathieu *et al.* compared registrations and publications of randomized controlled trials in journals with highest impact factors in cardiology, rheumatology and gastroenterology, finding that 31% or properly registered RCTs had discrepancies between registered and published primary outcomes[12]; according to Rayhill *et al.*, only about 25% of RCTs published in the core headache medicine journals displayed proper compliance with trial registration[13]. In two systematic reviews summarizing studies that compared registrations with full reports, Jones *et al.* found the median proportion of trials with identified discrepancy in primary outcome was 31%[14], and Li *et al.* also reported high level of inconsistency in outcome reporting ranging from 14% to 100%[11].

The ongoing pandemic of Corona Virus Disease 2019 (COVID-19) is a major public health concern. Numerous clinical trials have been registered and published to address scientific questions regarding the prevention, treatment and prognosis of the disease, and so far, many reports have been published. Kataoka *et al.* examined COVID-19 RCT articles in medRxiv and PubMed, revealing problems in research methods and the impact on report quality associated with accelerated publication[15]. However, no study has examined the transparency and selective reporting in COVID-19 trials by comparing full reports with trial registrations. In this study, we aimed to provide a comprehensive review

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of characteristics of registered COVID-19 related RCTs in mainland China, and evaluate the level of transparency and selective reporting by comparing trial registrations, published protocols and full reports.

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Methods

Clinical trial registration screening

International Clinical Trials Registry Platform (ICTRP), Chinese Clinical Trial Registry (ChiCTR), ClinicalTrials.gov (NCT), the ISRCTN registry (ISRCTN) and EU Clinical Trial Register (EUCTR) were accessed on 1-Feb-2022. A complete list of COVID-19 trials updated on 22-Jan-2022 was retrieved from ICTRP. Eligible studies were RCTs related to prevention, treatment or prognosis of COVID-19 infection. Studies were excluded if they were observational, non-randomized, single-arm trials or outside of mainland China. For multicenter trials, all centers must be located in mainland China to be eligible for analysis. From ChiCTR, index of studies of COVID-19 updated on 22-Dec-2021 was obtained. Studies registered after this date were screened manually. From NCT, list of interventional clinical studies related to COVID-19 was accessed, and map panel was used to select studies performed in mainland China. For ISRCTN and EUCTR, searches were conducted manually with keywords “COVID-19”, “covid19”, “SARS-Cov-2” or “2019-nCov” and the results were screened for eligibility according to above mentioned criteria (for review protocol and detailed search strategy, see Supplementary 1).

We extracted registration ID, date of registration, date of last update, date of first enrollment, scientific/official title, objective, intervention of interest, comparator, primary purpose, recruitment status, estimated enrollment, arms, ethical approval information (ChiCTR only), name and location of centers, randomization (ChiCTR only), masking, inclusion and exclusion criteria, primary outcomes and other relevant information from

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registration records,. Repeated registration was identified through examining similarity of trial characteristics. For multi-registered trials, only the record cited by publication and (if all/none of repeated registration records was cited) the most recently updated record shall be eligible for further analysis. Characteristics of included registrations were summarized and presented as count (%) or median (Interquartile range). Scatter plot and line chart were plotted to illustrate the trend of registered trials in each province of mainland China.

Literature search

Literatures citing these registrations were searched from 10-Feb-2022 to 12-Feb-2022 in PubMed, Embase, Cochrane Library, Google Scholar and two frequently used databases for Chinese language literature, CNKI.net and Wanfangdata, using registration ID as unique identifiers. From published protocols and trial reports, date of first enrollment, estimated enrollment, arms, center names, inclusion/exclusion criteria, primary/secondary outcomes and information regarding randomization and masking were extracted. Risk of bias assessment for published trial reports was performed using RoB 2[16]. Next, information extracted from registration records, published protocols and reports was compared, and the level of consistency was evaluated.

Registration screening, literature search, data extraction, risk of bias assessment and comparison were independently performed by two reviewers and consensus was reached through discussion.

Patient and Public Involvement

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Patients or public was not involved in the design, conduct, reporting, or dissemination plans, since the research question of this study made such involvement unnecessary.

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Results

A total of 435 registration records met eligibility criteria, in which 20 studies were repeatedly registered on ChiCTR and NCT, making the number of eligible studies 415 (see Supplementary 2). The flow chart of screening process is shown in ***Figure 1***.

Characteristics of registered trials

Among all included studies, 303 (73.0%) were registered in ChiCTR, 111 (26.7%) in NCT and 1 (0.2%) in ISRCTN and 243 (58.6%) were registered before the date of first enrollment. Most of the trials aimed to investigate the prevention (105, 25.3%) or treatment (290, 69.9%) of COVID-19. At the time of the most recent update, 133 (32.0%) trials did not start recruitment, 206 (49.6%) were recruiting and 55 (13.3%) trials completed recruitment, while 21 (5.1%) studies were suspended, terminated or withdrawn. The median estimated enrollment was 120. 32.8% trials (126) were multicentered, and 68.4% (284) were two-armed. For participants, 285 (68.7%) trials recruited clinical diagnosed COIVD-19 infected patients, 8 (1.9%) recruited suspected cases or close contacts of diagnosed patients, and 122 (29.4%) recruited participants not infected by COVID-19. 39.5% (164) registrations didn't specify any masking method, 125 (30.1%) were open-label studies and single/double masking was applied in 126 (30.4%) studies. The 415 trials registered a total of 2951 outcomes, including 1048 primary outcomes and 1903 secondary outcomes. 186 (44.8%) trials specified 1 primary outcome in registration record and 226 (54.5%) declared more than one primary outcomes. Primary outcome was missing for 3 (0.9%) registrations. 313 (75.4%) registrations had

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pre-specified secondary outcomes. (*Table 1*)

Charts were plotted to illustrate the trend of increasing trial registration in mainland China and each province. The first COVID-19 trial in mainland China was registered on 23-Jan-2020. As the pandemic began in Wuhan, COVID-19 related RCTs emerged in Hubei province first, and then expanded to other provinces. The next four months witnessed dramatic increase of trial registrations. This trend reached plateau around June 2020 and was thereafter steadily increasing until the day of data extraction (*Figure 2A*).

Publications from registered trials

From the 415 registered trials, 85 reports and 20 protocols were published; 8 reports were excluded from analysis since the study design was described as non-RCT (see Supplementary 1). Risk of bias assessment was performed for 77 RCT reports. Overall, high risk was found in 35 (45.5%) studies; 33 (42,9%) studies had some concerns; 9 (11.7%) were at low risk. The most common risk of bias was selection of reported result, with 60 (77.9%) studies having some concerns or high risk. Deviations from intended interventions (43, 55.9% for studies with some concerns or high risk, same below) and randomization issues (31, 40.3%) were also frequently documented. (*Figure 2B*)

Comparison between protocols and trial registrations

In 20 published protocols, 6 (30.0%) had deviation in sample size and 8 (40.0%) had discrepancy in setting; More than half of them differed in inclusion criteria (12, 60.0%) or exclusion criteria (10, 50.0%); 9 (45.0%) did not specify who was masked in the text or registration. There were 11 (55%) discrepancies between the primary outcomes in the

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registrations and the protocols. This included introducing new primary outcomes in protocol (8, 40.0%), omitting registered primary outcomes (8, 40.0%) and describing registered primary outcomes as secondary outcomes (9, 45.5%). None of the protocols declared outcome change and rationale. Only 2 (10.0%) protocols were consistent in all domains (*Table 2)* (see Supplementary 2).

Comparison between reports and trial registrations

From 74 registrations, 77 RCT reports eligible for analysis were published. 47 (61.0%) of the reports had discrepancies in sample size and the deviation was more than 20% in 36 (46.8%) of them. Discrepancy in setting was present in 21 (27.3%) reports. 54 (70.1%) reports had deviation in inclusion criteria and 46 (59.7%) had different exclusion criteria. 7 (9.1%) reports had deviation in masking method, among which masking was upgraded (open-label changed to single/double-blind, or single-blind changed to double-blind) in 5 (6.5%) studies and downgraded in 2 (2.6%) study. In primary outcome, 4 (5.2%) reports did not specify primary outcome in text or corresponding registration; 34 (44.2%) reports had deviation in primary outcome, including introducing new primary outcomes (18, 23.4%), omitting registered primary outcomes (23, 29.9%), describing registered secondary outcomes as primary outcomes (7, 9.1%) and describing registered primary outcomes as secondary outcomes (15, 19.5%). Furthermore, 7 (9.1%) reports did not specify time frame of primary outcome measurement in registry/text and 6 (7.8%) had deviation in time frame of primary outcome measurement. None of the reports declared outcome change and rationale. Only 4 (5.2%) reports maintained full consistency in all

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domains (***Table 2***). Among the 183 primary outcomes of the 74 registrations, 59 (32.2%) were correctly reported (see Supplementary 2).

Comparison between protocols and full reports

There were 7 trials where both protocol and RCT report were published. When comparing full reports to protocols, deviation in sample size and setting was found in 6 (85.7%) and 3 (42.9%) reports, respectively. More than half of the reports (4, 57.1%) differed from protocols in inclusion criteria and 3 (42.9%) differed in exclusion criteria. Deviation in masking was found in 1 (14.3%) report. 5 (71.4%) reports had difference in primary outcome or time frame of primary outcome. None of the reports disclosed outcome change and rationale. None of the reports maintained full consistency with the protocols (***Table 3***) (see Supplementary 2).

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Discussion

To summarize, 20 protocols and 77 RCT reports were published from 415 registrations in mainland China. Comparing to registrations, 90% protocols and 94.8% reports had discrepancy in at least one domain, in which deviation in primary outcome or time frame of primary outcome measurement occurred in 60% (12/20) protocols and 51.9% (40/77) reports. In trials where both protocol and report were published, 71.4% (5/7) full reports had discrepancies in primary outcome or time frame of measurement comparing to the protocols. Furthermore, risk of bias assessment revealed that a majority (88.4%) of the RCT reports had some concerns or high risk in overall bias, and selection of reported result was the most prevalent issue.

Since the pandemic of COVID-19 is a major public health issue, RCT evidence is urgently needed to support clinical decision making in the prevention, treatment or prognosis of this disease. However, methodological concerns in such trials were noted. Previously, Kataoka *et al.* reported research methods and reporting problems in medRxiv and PubMed publications related to COVID-19 RCTs, and inconsistency with trial registration was identified in 62% of 13 medRxiv literatures and 30% of 16 PubMed articles; the authors pointed out that these problems might result from the accelerated publication process[15], yet the study only covered a limited number of publications because it was performed in June 2020. One year and a half later, our comprehensive review of COVID-19 trial registrations, protocols and reports suggested the finding was not incidental.

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To assess the significance of our findings, we compared the results with other studies that evaluated the consistency among trial registrations, protocols and reports. In 2011, Dwan *et al.* systematically reviewed cohort studies comparing contents of trial registry entries or protocols with trial reports, and in 33% to 67% of RCTs primary outcome was the same in protocol as in publication, while in 69% to 82% of RCTs primary outcome in report was consistent with trial registration[17]; in the study of Li *et al.*, the median inconsistency of outcome reporting was 54% (IQR: 29% ~ 72%)[11]; Jones *et al.* reported a median proportion of 31% trials had discrepancy between registered and published primary outcome. The rates of discrepancy reported by our study were higher than Dwan *et al.* or Jones *et al.* studies, and were similar to the median rate reported by Li *et al.*

These results revealed compromised transparency in COVID-19 related RCT reporting in mainland China, and also suggested the presence of selective reporting in RCTs in other fields of study. Such findings are noteworthy especially for peer-reviewers and journal editors, as compromised transparency might undermine the value of these trials. Furthermore, the fact that RCT reports were published by less than a quarter of registered trials indicated potential presence of publication bias: a phenomenon of selective publication of studies depending on the results[18]. However, since our research methods were not designed to detect publication bias, we decided to leave this question to future research.

ICMJE's policy of mandatory prospective registration in member journals led to dramatic increase of trial registrations, while the registration data were often inadequate, changed

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over time and differed between registration and publication[19]. The goal of ICMJE's policy was to promote transparent reporting of trials, yet this could not be achieved unless journal editors and reviewers fully utilize information provided by trial registration: a survey by Mathieu *et al.* suggested only around 1/3 reviewers routinely used registration information when evaluating manuscripts[20]. Changes in primary outcome or other domains of trial design might be due to either good reasons or investigators' effort to produce favorable results from the data[14]. In either case, the validity of evidence provided by the trial could not be confidently assessed without emphasizing transparency of reporting. Cooperation of different stakeholders is required to promote transparency. We suggest principal investigators should ensure that the trial is prospectively registered and registration information is properly filled; peer reviewers should routinely use trial registries to assess manuscripts and demand explanation whenever discrepancy occurs; journal editors should prioritize the evaluation of trial registration in peer review process[11,14]. Future studies will be needed to reveal the trend of consistency over time and assess possible improvement caused by increasing scrutiny of peer reviewers, new policy of journals and ascensive familiarity of investigators with trial registration process.

The study had several limitations. First, the complete lists of COVID-19 related clinical trials were retrieved from ICTRP and ChiCTR and filtered by the labels provided by these lists, thus, the completeness and precision of record screening relied on the correctness of these lists. Second, the search of publications for eligible registration record was conducted with trial registration number as unique identifiers; if a publication did not cite any registration number, the article could not be included in analysis. This approach

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might lead to omission of publications that did not contain any trial identifier, but provided definitive evidence in linking registrations to publications and minimized the possibility of making mistakes. Third, the reviewers only analyzed the most recent updated version of registration record, without considering the historical changes, yet the validity of conclusion of this review is not jeopardized, since this approach tended to overestimate, but not underestimate, the overall transparency, in that registrations are often modified after trial completion to display false consistency with publications[14,19]. Last, our search results were limited to peer-reviewed English and Chinese literatures, and the findings could not be reliably represent studies published in languages other than English or Chinese. We did not review any literatures that had not undergone peer-review process (for example, manuscripts posted on preprint servers). However, without quality control in review process, it would be reasonable to assume that such preprints have no lower rate of discrepancy compared to peer-reviewed publications.

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Conclusion

The high rates of discrepancy among registrations, protocols and full reports of COVID-19 related RCTs in mainland China revealed compromised transparency in trial reporting. Investigators, peer reviewers and journal editors should make efforts to improve the utilization of trial registration information and promote transparent reporting.

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Contributorship Statement

Both reviewers (Y.C. and R.Y.) contributed equally in conceptualization, data extraction, analysis, visualization and drafting the manuscript. Both reviewers have read and approved this manuscript.

Data Availability Statement

Not applicable.

Transparency Statement

The authors affirm that the manuscript is an honest, accurate and transparent of the study reported and no important aspect of the study has been omitted.

Competing interest statement

None declared.

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Ethics Approval Statement

Ethics approval from institutional review board was exempted since the study did not involve human participants or animal subjects.

Registration

The systematic review was not prospectively registered.

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Figure Legend

Figure 1. *The screening process of registration records.*

Figure 2. (A) *The trend of registration of COVID-19 related RCT trials in total and in each province. When a trial is multicentered and recruited participants in multiple provinces, the trial is considered to take place in these provinces simultaneously. The publication status of these registered trials is also shown.* ***(B)*** *Results of risk of bias assessment (RoB2) are shown for each RCT report included in this review, the risks in each domain and overall risk are also summarized.*

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Table 1. *Characteristics of all included studies.*

Characteristic	Value	Proportion (%)
Count of Eligible Studies	415	100
Registry		
ChiCTR	303	73.0
Clinicaltrials.gov	111	26.7
ISRCTN	1	0.2
Registration Status		
Prospective	243	58.6
Retrospective	172	41.4
Primary Purpose		
Prevention	105	25.3
Treatment	290	69.9
Prognosis/Quality of Life	19	4.6
Health Services Research	1	0.2
Multicenter Research		
Yes	136	32.8
No	273	65.8
Unknown	6	1.4
Recruitment Status		
Not yet recruiting	133	32.0
Recruiting	206	49.6

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Completed	55	13.3
Suspended, Terminated or Withdrawn	21	5.1
Suspended Research	34	8.2
Estimated Enrollment		
Median (Interquartile Range)	120 (240)	
Min, Max	0, 29000	
Missing	3	0.9
Number of Arms		
2 Arms	284	68.4
3 Arms	59	14.2
4 Arms	31	7.5
5 Arms	5	1.2
6-10 Arms	25	6.0
More than 10 Arms	11	2.7
Participants		
Patient	285	68.7
Suspected or Close Contact of Confirmed Patient	8	1.9
Healthy	122	29.4
Masking		
Single/Double Blind	126	30.4
Open Label	125	30.1
Not Stated	164	39.5

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Count of Primary Outcome		
0	3	0.7
1	186	44.8
2	101	24.3
3	49	11.8
4	28	6.7
5	14	3.4
More than 5 Primary Outcomes	34	8.2
Median (Interquartile Range)	2 (2)	
Min, Max	0, 23	
Total Number of Primary Outcomes	1048	
Count of Secondary Outcome		
0	102	24.6
> 0	313	75.4
Median (Interquartile Range)	4 (6)	
Min, Max	0, 32	
Publication		
None	322	77.6
Protocol Only	13	3.1
Report Only	73	17.6
Both Protocol and Report	7	1.7

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Table 2. Comparison between registration records and publications.

Domain	Protocol N=20	Report N=77
Sample size		
Not specified in registry or publication	1 (5.0%)	0
Deviation (Any)	6 (30.0%)	47 (61.0%)
Deviation >= 1%	5 (25.0%)	47 (61.0%)
Deviation >= 20%	5 (25.0%)	36 (46.8%)
Deviation >= 50%	4 (20.0%)	25 (32.5%)
Deviation >= 100%	4 (20.0%)	2 (2.6%)
Setting (Center)		
Not specified in registry or publication	5 (25.0%)	22 (28.6%)
Deviation (Any)	8 (40.0%)	21 (27.3%)
Inclusion Criteria		
Not specified in registry or publication	0	3 (3.9%)
Deviation (Any)	12 (60.0%)	54 (70.1%)
Exclusion Criteria		
Not specified in registry or publication	0	8 (10.4%)
Deviation (Any)	10 (50.0%)	46 (59.7%)
Masking		
Not specified in registry or publication	9 (45.0%)	28 (36.4%)
Deviation (Any)	0	7 (9.1%)

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Domain	Protocol N=20	Report N=77
Masking up-graded in publication	0	5 (6.5%)
Masking down-graded in publication	0	2 (2.6%)
Primary Outcome		
Not specified in registry or publication	0	4 (5.2%)
Deviation in nature of primary outcome (Any)	11 (55.0%)	34 (44.2%)
New primary outcome introduced in publication	8 (40.0%)	18 (23.4%)
Registered primary outcome omitted in publication	8 (40.0%)	23 (29.9%)
Secondary outcome in registry described as primary outcome in publication	0	7 (9.1%)
Primary outcome in registry described as secondary outcome in publication	9 (45.0%)	15 (19.5%)
Not specified time frame of primary outcome in registry or publication (if no deviation in nature of primary outcome)	2 (10.0%)	7 (9.1%)
Deviation in time frame of primary outcome in registry or publication (if no deviation in nature of primary outcome)	1 (5.0%)	6 (7.8%)

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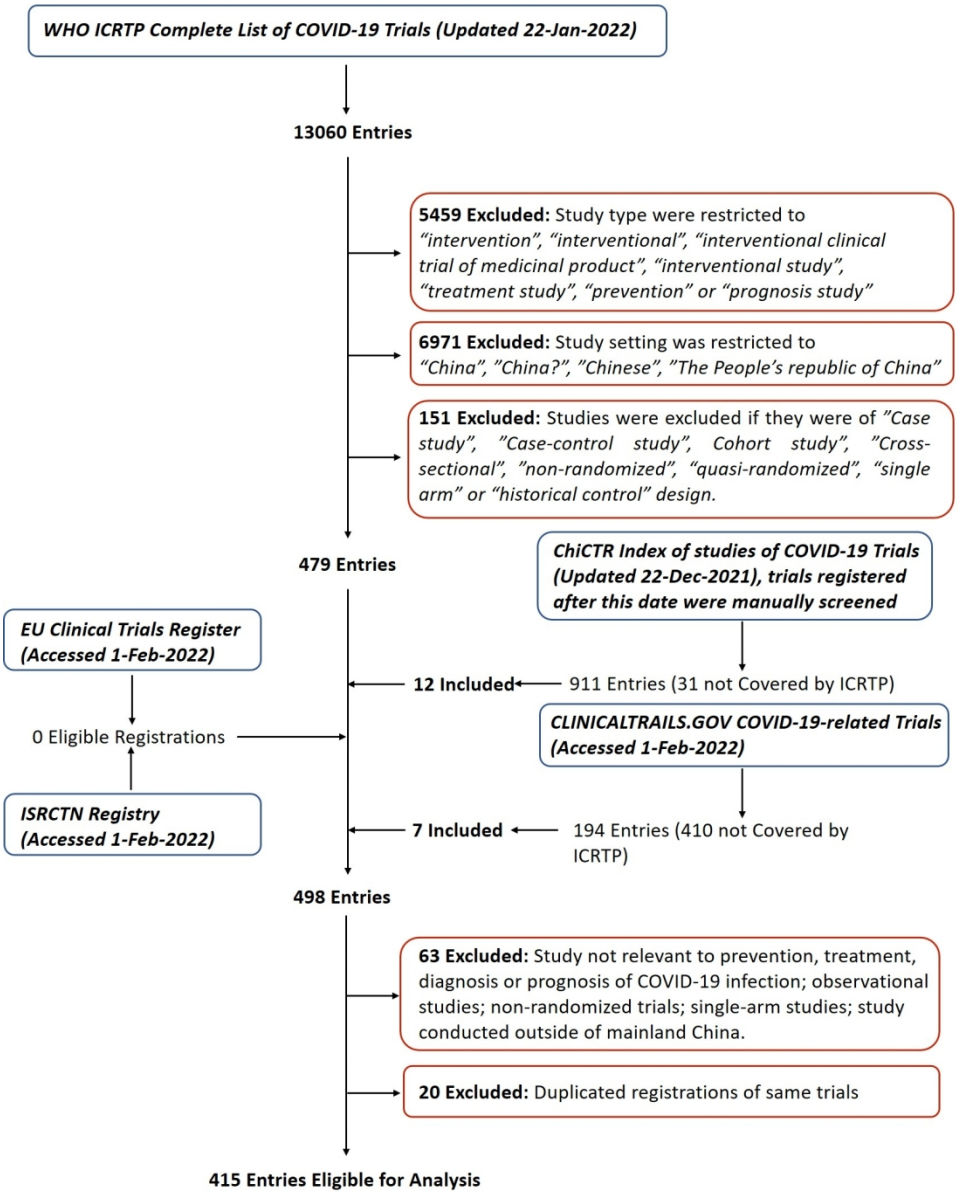
Table 3. Comparison between protocols and full reports.

Domain	RCT full reports N=7
Sample size	
Not specified in protocol or report	0
Deviation (Any)	6 (85.7%)
Deviation >= 1%	6 (85.7%)
Deviation >= 20%	5 (71.4%)
Deviation >= 50%	3 (42.9%)
Deviation >= 100%	0
Setting (Center)	
Not specified in protocol or report	2 (28.6%)
Deviation (Any)	3 (42.9%)
Inclusion Criteria	
Not specified in protocol or report	0
Deviation (Any)	4 (57.1%)
Exclusion Criteria	
Not specified in protocol or report	0
Deviation (Any)	3 (42.9%)
Masking	
Not specified in protocol or report	1 (14.3%)
Deviation (Any)	1 (14.3%)

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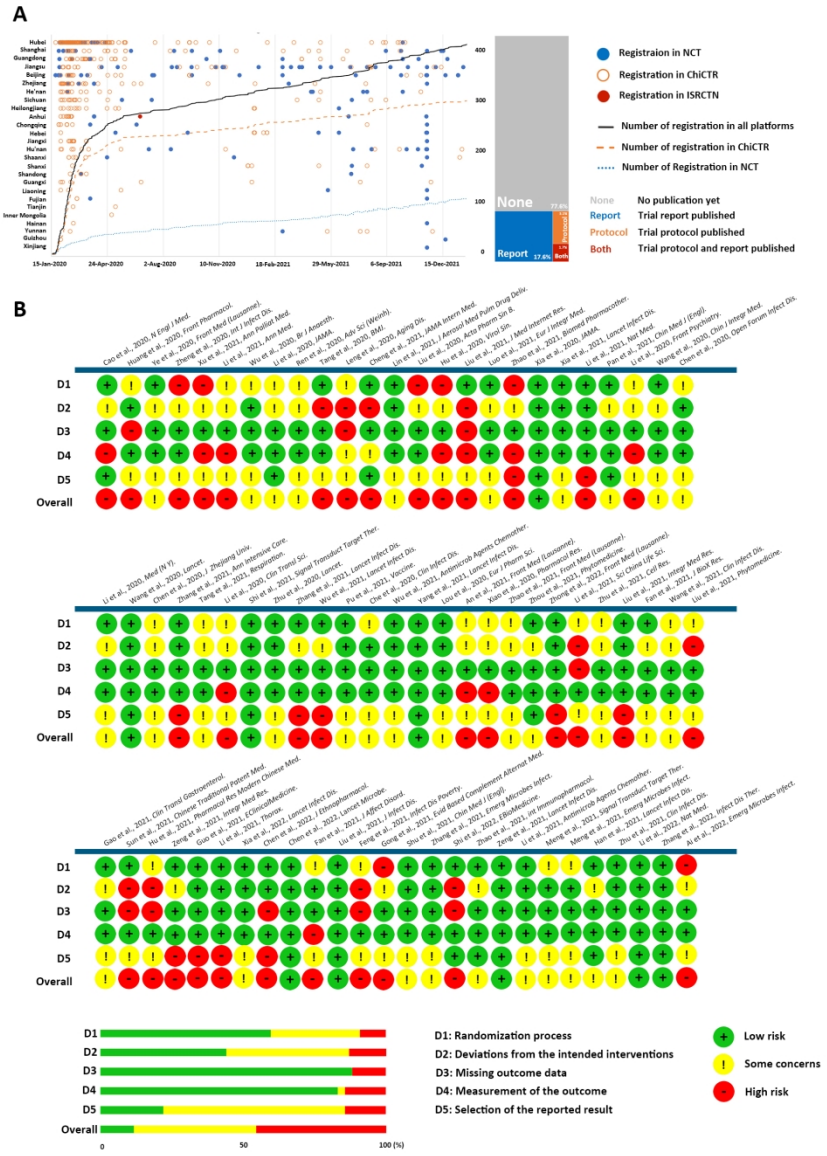
Domain	RCT full reports N=7
Masking up-graded in report	1 (14.3%)
Masking down-graded in report	0
Primary Outcome	
Not specified in protocol or report	0
Deviation in nature of primary outcome (Any)	3 (42.9%)
New primary outcome introduced in report	1 (14.3%)
Registered primary outcome omitted in report	1 (14.3%)
Secondary outcome in protocol described as primary outcome in report	1 (14.3%)
Primary outcome in protocol described as secondary outcome in report	2 (28.6%)
Not specified time frame of primary outcome in protocol or report (if no deviation in nature of primary outcome)	0
Deviation in time frame of primary outcome in protocol or report (if no deviation in nature of primary outcome)	2 (28.6%)

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The screening process of registration records.

290x359mm (150 x 150 DPI)



(A) The trend of registration of COVID-19 related RCT trials in total and in each province. When a trial is multicentered and recruited participants in multiple provinces, the trial is considered to take place in these provinces simultaneously. The publication status of these registered trials is also shown. (B) Results of risk of bias assessment (RoB2) are shown for each RCT report included in this review, the risks in each domain and overall risk are also summarized.

687x978mm (72 x 72 DPI)

Review Protocol

Objective

To evaluate the transparency of COVID-19 related RCT reporting in mainland China through comparing trial registrations with publications.

Ethics Approval

The study is exempt from ethics approval because only publicly available databases and registries will be used as data source. No human participants and animal subjects will be involved in the study.

Eligibility Criteria for Registrations

Registered randomized controlled trials related to prevention, treatment or prognosis of COVID-19 in mainland China will be included. Case report, case series, cross-sectional study, case-control study, cohort study, survey and other observational studies will be excluded. Studies will be excluded if randomization is not used or without a control group. For multicenter trials, all registered centers must be within mainland China to meet eligibility criteria.

Data Source and Search Strategy for Registrations

International Clinical Trials Registry Platform (ICTRP), Chinese Clinical Trial Registry (ChiCTR), ClinicalTrials.gov (NCT), the ISRCTN registry (ISRCTN) and EU Clinical Trial Register (EUCTR) will be searched. For ICTRP and ChiCTR, complete lists of COVID-19 clinical trial registrations will be downloaded. Filters will be applied to identify studies which meet eligibility criteria. For NCT, the page listing COVID-19 studies will be accessed and eligible studies will be identified using website filters and the map panel. ISRTCN and EUCTR will be manually searched with the keywords “COVID-19”, “Sars-Cov-2”, “covid19” and “2019-nCov”. All search results will be examined manually to ensure their eligibility.

Data Source and Search Strategy for Publications

PubMed, Embase, Cochrane Library, CNKI.net and Wanfangdata will be searched using trial

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registration IDs of eligible trials. Only publications in English and Chinese will be included.

Data Extraction

From trial registrations, registration ID, date of registration/first submission, date of last update, date of first enrollment, scientific/official titles, primary purpose, recruitment status, intervention, source of funding, primary sponsor, ethics approval information, setting, randomization and masking methods, inclusion and exclusion criteria, primary/secondary outcomes and time frame of outcome measurement will be extracted.

From publications, title, estimated/actual enrollment, center name, inclusion/exclusion criteria, masking, primary outcomes and time frame of outcome measurement will be extracted.

Risk of Bias Assessment

Risk of bias assessment will be performed with RoB 2 for full reports of trials.

Data Synthesis

The screening process of registrations will be presented with a flow diagram. Characteristics of included trial registrations will be presented with descriptive statistics, in count and proportion for categorical data, or with median, max value, minimum value and interquartile range for quantitative data. The trend of registrations from early 2020 will be presented with line chart. Estimated/actual enrollment, center name, inclusion/exclusion criteria, masking method, primary outcome and time frame of primary outcome measurement information extracted from trial registrations and publications will be compared, and count and percentage of inconsistency within each domain will be presented. Risk of bias assessment results will be presented with figure.

Review Process

The screening process, data extraction, risk of bias assessment, registration-publication comparison will be independently completed by two reviewers, and the results will be compared. Disagreements will be resolved through discussion.

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Protocol Amendments

May-2021

For trials which are repeatedly registered on two or more clinical trial registries, if they have any publication, the registration not cited by publication will be excluded from the analysis. If all or none of the repeated registrations are cited by publications, the record with most recent update time will be included in the analysis. Repeated registrations will be detected by reviewing the title, objective and name of principle investigator.

Publications citing an eligible trial registration identifier but declared to be of non-RCT design will be excluded from the analysis. The number of such publications will be reported.

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Google Scholar will be searched for publications to ensure the completeness of search results.

Information extracted from protocols and corresponding full reports will also be compared to evaluate protocol-report consistency.

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Search Strategy

Clinical Trial Registration

International Clinical Trials Registry Platform (ICTRP, https://www.who.int/clinical-trials-registry-platform) was accessed and a list of COVID-19 trials (updated on 22-Jan-2022) in csv format was downloaded. The file was opened with Microsoft Excel. The following filters were applied to "Study type" column: "intervention" "interventional" "interventional clinical trial of medicinal product" "interventional study" "treatment study" "prevention" "prognosis study". The following filters were applied to "Countries" column: "China" "China?" "Chinese" "The People's republic of China". From the "Study design" column, "Case study" "Case-control study" "Cohort study" "Cross-sectional" studies, non-randomized/quasi-randomized studies and studies with single arm or historical control were excluded.

Chinese Clinical Trial Registry (ChiCTR, http://www.chictr.org.cn/enIndex.aspx) was accessed and index of studies of COVID-19 (updated on 22-Dec-2021) in csv format was downloaded. The ChiCTR index was then mapped to the ICTRP COVID-19 trials list to identify any studies listed in ChiCTR but not in ICTRP. Studies registered after 22-Dec-2021 were screened manually.

List of COVID-19 related studies from ClinicalTrials.gov was accessed (https://clinicaltrials.gov/ct2/results?cond=COVID-19). The filter "Study type - Interventional (Clinical Trial)" was applied, and studies registered in mainland China were identified using the "On Map" panel. The listed studies were downloaded and compared with ICTRP records in case of omissions.

In ISRCTN registry (ISRCTN, https://www.isrctn.com/) and EU Clinical Trial Register (EUCTR, https://www.clinicaltrialsregister.eu/), the registry was searched with the following search string: "covid19 or COVID-19 or SARS-Cov-2 or 2019-nCov", and the country of recruitment was set to "China" and "Outside EU/EEA", respectively. Search results were also compared with ICTRP records.

All above-mentioned registries were accessed on 1-Feb-2022.

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Publication

Search was performed in PubMed (https://pubmed.ncbi.nlm.nih.gov/), Embase (https://www.embase.com/), Cochrane Library (https://www.cochranelibrary.com/search), Google Scholar (https://scholar.google.com/), CNKI.net (https://www.cnki.net/) and Wanfangdata (https://www.wanfangdata.com.cn/) using the trial registration number with exact match method.

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Literatures

This systematic review identified 85 reports[1-85] and 20[86-105] protocols from 415 clinical trial registration records. For further analysis, 8 reports[78-85] were excluded because non-RCT study design was adopted.

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Non-RCT Publications	Registry Platform	Date of Registration/Fi	Last updated post
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	ClinicalTrials.gov	2020/1/23	6/16/2020
	ClinicalTrials.gov	2020/1/28	5/5/2020
	ChiCTR	2020/1/29	3/2/2020
	ChiCTR	2020/1/29	3/2/2020
	ClinicalTrials.gov	2020/1/29	4/13/2020
	ChiCTR	2020/1/30	2/12/2020
	ClinicalTrials.gov	2020/1/30	3/17/2020
	ClinicalTrials.gov	2020/1/30	7/1/2020

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ClinicalTrials.gov	2020/1/31	4/15/2020
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ChiCTR	2020/2/16	2/17/2020
ChiCTR	2020/2/16	2/17/2020
ChiCTR	2020/2/17	2/17/2020
ChiCTR	2020/2/17	3/17/2020
ChiCTR	2020/2/17	7/22/2021
ChiCTR	2020/2/18	2/18/2020

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Report-902	ChiCTR	2020/2/18	3/17/2020
	ChiCTR	2020/2/18	2/18/2020
	ChiCTR	2020/2/18	2/18/2020
	ChiCTR	2020/2/18	2/18/2020
	ChiCTR	2020/2/18	3/16/2020
	ChiCTR	2020/2/18	2/19/2020
	ChiCTR	2020/2/18	3/9/2020
	ChiCTR	2020/2/18	2/19/2020
	ChiCTR	2020/2/19	6/17/2020
	ChiCTR	2020/2/19	2/24/2020

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ChiCTR	2020/2/19	2/19/2020
ChiCTR	2020/2/19	2/19/2020
ChiCTR	2020/2/19	2/28/2020
ChiCTR	2020/2/19	2/19/2020
ChiCTR	2020/2/19	2/20/2020
ClinicalTrials.gov	2020/2/19	7/7/2021
ChiCTR	2020/2/20	2/21/2020
ChiCTR	2020/2/20	3/5/2020
ChiCTR	2020/2/20	3/1/2020
ChiCTR	2020/2/20	3/24/2020

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ChiCTR	2020/2/21	3/16/2020
ChiCTR	2020/2/21	2/21/2020
ChiCTR	2020/2/21	2/21/2020
ChiCTR	2020/2/21	2/21/2020
ClinicalTrials.gov	2020/2/21	3/17/2020
ChiCTR	2020/2/22	2/22/2020
ChiCTR	2020/2/22	3/16/2020
ChiCTR	2020/2/22	2/22/2020
ChiCTR	2020/2/22	3/15/2020
ChiCTR	2020/2/22	2/22/2020

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ChiCTR	2020/2/22	2/23/2020
ChiCTR	2020/2/22	2/23/2020
ChiCTR	2020/2/23	2/23/2020
ChiCTR	2020/2/23	2/23/2020
ChiCTR	2020/2/23	2/23/2020
ChiCTR	2020/2/23	3/17/2020
ChiCTR	2020/2/24	3/14/2020
ChiCTR	2020/2/24	2/24/2020
ChiCTR	2020/2/24	3/17/2020
ChiCTR	2020/2/24	2/24/2020

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ChiCTR	2020/2/24	2/24/2020
ChiCTR	2020/2/24	2/24/2020
ChiCTR	2020/2/24	3/16/2020
ChiCTR	2020/2/24	3/17/2020
ClinicalTrials.gov	2020/2/24	8/19/2020
ClinicalTrials.gov	2020/2/24	3/18/2020
ChiCTR	2020/2/25	3/27/2020
ChiCTR	2020/2/25	3/17/2020
ChiCTR	2020/2/25	2/26/2020
ChiCTR	2020/2/26	3/17/2020

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ChiCTR	2020/2/26	2/26/2020
ChiCTR	2020/2/26	3/23/2020
ChiCTR	2020/2/26	2/26/2020
ChiCTR	2020/2/26	8/4/2020
ChiCTR	2020/2/26	2/26/2020
ChiCTR	2020/2/26	2/26/2020
ChiCTR	2020/2/26	2/27/2020
ChiCTR	2020/2/27	1/7/2022
ChiCTR	2020/2/28	2/29/2020
ChiCTR	2020/2/28	2/28/2020

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ChiCTR	2020/2/28	4/10/2020
ChiCTR	2020/2/28	3/14/2020
ClinicalTrials.gov	2020/2/28	7/9/2020
ChiCTR	2020/2/29	7/9/2020
ChiCTR	2020/3/1	4/19/2021
ChiCTR	2020/3/1	3/1/2020
ChiCTR	2020/3/1	3/13/2020
ChiCTR	2020/3/1	3/3/2020
ChiCTR	2020/3/1	3/20/2020
ChiCTR	2020/3/1	3/1/2020

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ChiCTR	2020/3/2	3/16/2020
ChiCTR	2020/3/2	3/2/2020
ChiCTR	2020/3/2	3/17/2020
ChiCTR	2020/3/2	3/8/2020
ChiCTR	2020/3/3	7/9/2020
ChiCTR	2020/3/3	3/13/2020
ChiCTR	2020/3/3	3/16/2020
ChiCTR	2020/3/3	3/3/2020
ChiCTR	2020/3/3	3/3/2020
ChiCTR	2020/3/3	3/14/2020

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ChiCTR	2020/3/3	3/3/2020
ClinicalTrials.gov	2020/3/4	3/16/2020
ChiCTR	2020/3/5	3/14/2020
ChiCTR	2020/3/5	3/5/2020
ChiCTR	2020/3/6	10/5/2020
ChiCTR	2020/3/6	3/6/2020
ChiCTR	2020/3/6	3/7/2020
ChiCTR	2020/3/6	3/16/2020
ChiCTR	2020/3/8	3/8/2020
ChiCTR	2020/3/8	3/15/2020

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Report-903	ChiCTR	2020/3/8	3/8/2020
	ClinicalTrials.gov	2020/3/9	4/10/2020
	ChiCTR	2020/3/9	3/9/2020
	ClinicalTrials.gov	2020/3/9	3/26/2020
	ChiCTR	2020/3/10	3/10/2020
	ChiCTR	2020/3/10	3/10/2020
	ChiCTR	2020/3/10	3/10/2020
	ChiCTR	2020/3/10	3/16/2020
	ChiCTR	2020/3/10	3/26/2020
	ChiCTR	2020/3/11	3/12/2020

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ChiCTR	2020/3/11	3/12/2020
ChiCTR	2020/3/13	4/12/2020
ChiCTR	2020/3/13	3/16/2020
ChiCTR	2020/3/14	3/14/2020
ChiCTR	2020/3/15	3/16/2020
ClinicalTrials.gov	2020/3/15	7/7/2021
ChiCTR	2020/3/16	3/16/2020
ChiCTR	2020/3/16	3/16/2020
ChiCTR	2020/3/17	3/23/2020
ChiCTR	2020/3/17	3/17/2020

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ChiCTR	2020/3/17	3/23/2020
ChiCTR	2020/3/17	3/18/2020
ChiCTR	2020/3/18	3/18/2020
ChiCTR	2020/3/18	3/23/2020
ChiCTR	2020/3/18	3/19/2020
ClinicalTrials.gov	2020/3/20	3/24/2020
ChiCTR	2020/3/20	3/23/2020
ChiCTR	2020/3/20	3/20/2020
ChiCTR	2020/3/22	3/22/2020
ChiCTR	2020/3/22	3/23/2020

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ChiCTR	2020/3/23	5/4/2020
ClinicalTrials.gov	2020/3/23	4/15/2020
ChiCTR	2020/3/24	3/24/2020
ClinicalTrials.gov	2020/3/25	4/7/2020
ClinicalTrials.gov	2020/3/25	11/17/2020
ClinicalTrials.gov	2020/3/25	5/1/2020
ChiCTR	2020/3/26	3/26/2020
ChiCTR	2020/3/27	3/27/2020
ClinicalTrials.gov	2020/3/28	3/10/2021
ChiCTR	2020/4/1	4/1/2020

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ChiCTR	2020/4/1	4/1/2020
ClinicalTrials.gov	2020/4/1	4/24/2020
ClinicalTrials.gov	2020/4/1	4/24/2020
ChiCTR	2020/4/3	4/3/2020
ChiCTR	2020/4/6	9/14/2020
ClinicalTrials.gov	2020/4/7	5/18/2020
ChiCTR	2020/4/8	4/8/2020
ChiCTR	2020/4/11	4/13/2020
ChiCTR	2020/4/11	4/11/2020
ChiCTR	2020/4/13	4/13/2020

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	ClinicalTrials.gov	2020/4/14	7/29/2021
	ChiCTR	2020/4/15	4/15/2020
Report-911	ChiCTR	2020/4/16	9/21/2020
	ChiCTR	2020/4/17	4/17/2020
	ChiCTR	2020/4/20	4/27/2020
	ClinicalTrials.gov	2020/4/20	11/24/2021
	ChiCTR	2020/4/21	8/31/2020
	ChiCTR	2020/4/23	4/23/2020
	ChiCTR	2020/4/23	4/23/2020
	ChiCTR	2020/4/24	4/24/2020

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ClinicalTrials.gov	2020/4/24	4/28/2020
ChiCTR	2020/4/25	4/26/2020
ChiCTR	2020/4/25	4/25/2020
ChiCTR	2020/4/26	7/9/2020
ChiCTR	2020/4/27	4/27/2020
ChiCTR	2020/4/29	2/1/2021
ChiCTR	2020/4/29	8/14/2020
ChiCTR	2020/4/29	4/29/2020
ChiCTR	2020/5/2	1/25/2021
ChiCTR	2020/5/4	5/10/2020

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ChiCTR	2020/5/8	5/8/2020
ChiCTR	2020/5/9	5/9/2020
ClinicalTrials.gov	2020/5/9	8/18/2021
ClinicalTrials.gov	2020/5/13	7/21/2020
ChiCTR	2020/5/15	5/15/2020
ChiCTR	2020/5/15	7/9/2020
ChiCTR	2020/5/19	5/19/2020
ChiCTR	2020/5/21	5/25/2020
ClinicalTrials.gov	2020/5/21	6/4/2020
ChiCTR	2020/5/25	6/1/2020

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	ChiCTR	2020/5/26	6/1/2020
	ChiCTR	2020/5/26	5/26/2020
	ChiCTR	2020/5/27	5/28/2020
Report-904	ChiCTR	2020/5/27	5/28/2020
	ClinicalTrials.gov	2020/6/6	6/22/2020
	ChiCTR	2020/6/11	6/11/2020
	ClinicalTrials.gov	2020/6/16	12/3/2020
	ISRCTN	2020/6/22	11/17/2020
	ChiCTR	2020/6/24	7/9/2020
	ClinicalTrials.gov	2020/7/8	12/3/2020

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	ClinicalTrials.gov	2020/7/13	7/14/2020
	ClinicalTrials.gov	2020/7/14	4/12/2021
	ClinicalTrials.gov	2020/7/14	4/12/2021
	ClinicalTrials.gov	2020/7/17	3/23/2021
Report-905	ChiCTR	2020/7/19	7/20/2020
	ChiCTR	2020/8/15	8/30/2020
	ClinicalTrials.gov	2020/8/16	1/27/2021
	ClinicalTrials.gov	2020/8/18	11/11/2021
	ClinicalTrials.gov	2020/8/22	2/26/2021
	ClinicalTrials.gov	2020/8/27	2/11/2021

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ChiCTR	2020/8/29	10/26/2020
ChiCTR	2020/9/1	9/8/2020
ChiCTR	2020/9/11	1/20/2021
ClinicalTrials.gov	2020/9/15	11/9/2020
ChiCTR	2020/9/18	12/14/2020
ChiCTR	2020/9/19	12/14/2020
ClinicalTrials.gov	2020/9/22	11/27/2020
ClinicalTrials.gov	2020/9/23	12/28/2021
ChiCTR	2020/9/30	9/30/2020
ChiCTR	2020/10/2	12/21/2020

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ClinicalTrials.gov	2020/10/19	10/22/2020
ChiCTR	2020/10/22	1/19/2021
ChiCTR	2020/10/24	1/27/2021
ChiCTR	2020/10/28	1/30/2021
ClinicalTrials.gov	2020/11/4	7/27/2021
ChiCTR	2020/11/6	2/11/2021
ClinicalTrials.gov	2020/11/12	2/26/2021
ClinicalTrials.gov	2020/11/17	2/15/2021
ClinicalTrials.gov	2020/11/17	1/8/2021
ClinicalTrials.gov	2020/11/19	3/30/2021

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ChiCTR	2020/11/22	11/22/2021
ChiCTR	2020/11/22	11/22/2020
ClinicalTrials.gov	2020/12/7	12/11/2020
ChiCTR	2020/12/11	4/1/2021
ClinicalTrials.gov	2020/12/17	1/27/2021
ChiCTR	2021/1/1	3/21/2021
ChiCTR	2021/1/1	3/21/2021
ChiCTR	2021/1/1	3/21/2021
ChiCTR	2021/1/8	3/26/2021
ChiCTR	2021/1/12	1/14/2022

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ChiCTR	2021/1/12	1/13/2022
ChiCTR	2021/1/13	1/14/2022
ChiCTR	2021/1/14	4/9/2021
ClinicalTrials.gov	2021/1/20	5/25/2021
ChiCTR	2021/1/29	5/11/2021
ChiCTR	2021/2/4	2/4/2021
ChiCTR	2021/2/5	5/22/2021
ClinicalTrials.gov	2021/3/4	1/25/2022
ChiCTR	2021/3/5	7/5/2021
ClinicalTrials.gov	2021/3/15	7/26/2021

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ClinicalTrials.gov	2021/3/19	5/25/2021
ChiCTR	2021/3/30	10/24/2021
ClinicalTrials.gov	2021/4/3	8/11/2021
ChiCTR	2021/4/7	11/8/2021
ChiCTR	2021/4/7	11/8/2021
ChiCTR	2021/4/7	11/8/2021
ClinicalTrials.gov	2021/4/8	8/2/2021
ClinicalTrials.gov	2021/4/24	5/7/2021
ClinicalTrials.gov	2021/4/27	5/13/2021
ChiCTR	2021/5/1	12/13/2021

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ClinicalTrials.gov	2021/5/7	9/24/2021
ChiCTR	2021/5/8	12/17/2021
ChiCTR	2021/5/11	12/19/2021
ChiCTR	2/4/2021	5/20/2021
ClinicalTrials.gov	5/24/2021	11/8/2021
ClinicalTrials.gov	5/30/2021	6/8/2021
ClinicalTrials.gov	5/10/2021	6/30/2021
ChiCTR	7/5/2021	7/5/2021
ChiCTR	6/27/2021	1/16/2022
ChiCTR	6/20/2021	1/14/2022

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ClinicalTrials.gov	7/6/2021	7/8/2021
ClinicalTrials.gov	6/30/2021	7/22/2021
ClinicalTrials.gov	7/5/2021	7/27/2021
ClinicalTrials.gov	7/8/2021	7/23/2021
ClinicalTrials.gov	7/26/2021	8/3/2021
ClinicalTrials.gov	8/2/2021	8/4/2021
ClinicalTrials.gov	8/2/2021	8/6/2021
ClinicalTrials.gov	8/10/2021	8/18/2021
ClinicalTrials.gov	8/10/2021	8/18/2021
ClinicalTrials.gov	5/10/2021	8/18/2021

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ClinicalTrials.gov	8/17/2021	8/20/2021
ChiCTR	8/16/2021	8/16/2021
ChiCTR	8/10/2021	8/10/2021
ChiCTR	8/8/2021	8/30/2021
ChiCTR	8/8/2021	8/30/2021
ChiCTR	8/5/2021	8/5/2021
ChiCTR	8/2/2021	8/2/2021
ChiCTR	8/2/2021	8/2/2021
ChiCTR	8/2/2021	8/2/2021
ChiCTR	7/31/2021	7/31/2021

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ChiCTR	7/21/2021	7/21/2021
ChiCTR	9/4/2021	9/4/2021
ClinicalTrials.gov	2/6/2020	9/8/2021
ClinicalTrials.gov	9/5/2021	9/9/2021
ChiCTR	3/24/2021	9/19/2021
ClinicalTrials.gov	9/13/2021	9/16/2021
ChiCTR	9/26/2021	9/26/2021
ClinicalTrials.gov	7/3/2021	9/29/2021
ClinicalTrials.gov	7/13/2021	9/24/2021
ClinicalTrials.gov	10/5/2021	10/15/2021

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ClinicalTrials.gov	6/13/2021	10/21/2021
ClinicalTrials.gov	10/14/2021	10/25/2021
ClinicalTrials.gov	10/12/2021	10/25/2021
ChiCTR	10/9/2021	10/10/2021
ChiCTR	9/30/2021	10/3/2021
ChiCTR	9/29/2021	10/3/2021
ChiCTR	11/6/2021	11/6/2021
ChiCTR	11/6/2021	11/6/2021
ChiCTR	9/30/2021	11/5/2021
ClinicalTrials.gov	11/1/2021	11/4/2021

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ChiCTR	11/24/2021	11/30/2021
ClinicalTrials.gov	11/2/2021	11/23/2021
ChiCTR	8/9/2021	12/7/2021
ChiCTR	4/20/2021	12/6/2021
ClinicalTrials.gov	11/8/2021	12/14/2021
ClinicalTrials.gov	12/3/2021	12/28/2021
ClinicalTrials.gov	12/20/2021	12/21/2021
ClinicalTrials.gov	12/20/2021	12/21/2021
ClinicalTrials.gov	12/24/2021	12/29/2021
ChiCTR	5/23/2021	1/2/2022

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ClinicalTrials.gov	12/13/2021	1/20/2022
ClinicalTrials.gov	1/12/2022	1/31/2022
ClinicalTrials.gov	1/12/2022	1/31/2022
ChiCTR	8/31/2021	11/4/2021
ChiCTR	9/5/2021	9/7/2021
ChiCTR	10/12/2021	10/12/2021
ChiCTR	11/27/2021	11/27/2021
ChiCTR	11/30/2021	11/30/2021
ChiCTR	12/13/2021	12/13/2021
ClinicalTrials.gov	1/21/2022	1/24/2022

For peer review only

ClinicalTrials.gov	12/16/2021	1/19/2022
ClinicalTrials.gov	10/2/2021	2/1/2022
ClinicalTrials.gov	11/16/2021	1/3/2022
ClinicalTrials.gov	11/17/2021	1/11/2022
ChiCTR	2022/1/15	1/15/2022
ChiCTR	2022/1/19	1/19/2022

For peer review only

Scientific/Official Title

A randomized, controlled open-label trial to evaluate the efficacy and safety of lopinavir-ritonavir in hospitalized patients with 2019-nCoV infection

Glucocorticoid Therapy for Critically Ill Patients With Severe Acute Respiratory Infections Caused by COVID-19: a Prospective, Randomized Controlled Trial

Effects of Traditional Chinese Medicines (TCMs) on Patients With COVID-19 Infection: A Perspective, Open-labeled, Randomized, Controlled Trial

A study on treatment strategies of novel coronavirus pnueumonia patients

Comparative effectiveness and safety of ribavirin plus interferon-alpha, lopinavir/ritonavir plus interferon-alpha and ribavirin plus lopinavir/ritonavir plus interferon-alphain in patients with mild to moderate novel coronavirus pneumonia

Efficacy and Safety of Darunavir and Cobicistat for Treatment of COVID-19

Chinese Herbal medicine for Severe nevel coronavirus pneumonia (COVID-19): a Randomized Controlled Trial

Washed Microbiota Transplantation for Patients With 2019-nCoV Infection: a Randomized, Double-blind, Placebo-controlled Study

A Randomized, Open-label, Controlled Study of the Efficacy of Lopinavir Plus Ritonavir and Arbidol for Treating With Patients With Novel Coronavirus Infection

For peer review only

A Phase 3 Randomized, Double-blind, Placebo-controlled Multicenter Study to Evaluate the Efficacy and Safety of Remdesivir in Hospitalized Adult Patients With Mild and Moderate COVID-19.

A Phase 3 Randomized, Double-blind, Placebo-controlled, Multicenter Study to Evaluate the Efficacy and Safety of Remdesivir in Hospitalized Adult Patients With Severe COVID-19.

Clinical study of type I macrophages therapy in the treatment of novel coronavirus pneumonia (COVID-19)

A randomized, open-label, blank-controlled trial for Lian-Hua Qing-Wen Capsule/Granule in the treatment of suspected novel coronavirus pneumonia (COVID-19)

A randomized, open-label, blank-controlled trial for Lian-Hua Qing-Wen Capsule /Granule in the treatment of novel coronavirus pneumonia (COVID-19)

Randomized controlled trial for traditional Chinese medicine in the prevention of novel coronavirus pneumonia (COVID-19) in high risk population

A randomized controlled trial of integrated TCM and Western Medicine in the treatment of severe novel coronavirus pneumonia (COVID-19)

Combination of traditional chinese medicne and western medicine in the treatment of common type novel coronavirus pneumonia (COVID-19)

Pulmonary rehabilitation to improve pulmonary function and quality of life in patients with novel coronavirus pneumonia (COVID-19) in rehabilitation period

The effect of shadowboxing for pulmonary function and quality of life in patients with novel coronavirus pneumonia (COVID-19) in rehabilitation period

For peer review only

A Randomized Controlled Trial for Integrated Traditional Chinese Medicine and Western Medicine in the Treatment of Common Type Novel Coronavirus Pneumonia (COVID-19)

Traditional Chinese Medicine for Pulmonary Fibrosis, Pulmonary Function and Quality of Life in Patients With Novel Coronavirus Pneumonia (COVID-19) in Convalescent Period: a Randomized Controlled Trial

Traditional Chinese Medicine, Psychological Intervention and Investigation of Mental Health for Patients With Novel Coronavirus Pneumonia (COVID-19) in Convalescent Period

An Open, Prospective/Retrospective, Randomized Controlled Cohort Study to Compare the Efficacy of Two Therapeutic Schemes(Abidol Hydrochloride,Abidol Hydrochloride Combined With Interferon Atomization)in the Treatment of 2019-nCoV Pneumonia.

An Open, Prospective/Retrospective, Randomized Controlled Cohort Study to Compare the Efficacy of Three Antiviral Drugs(Abidol Hydrochloride, Oseltamivir and Lopinavir/Ritonavir) in the Treatment of 2019-nCoV Pneumonia.

An Open, Prospective/Retrospective, Randomized Controlled Cohort Study to Compare the Efficacy of Different Hormone Doses in the Treatment of 2019-nCoV Severe Pneumonia

A randomized, open label, parallel controlled trial for evaluating the efficacy of recombinant cytokine gene-derived protein injection on clearing novel coronavirus in patients with novel coronavirus pneumonia (COVID-19)

Chinese medicine prevention and treatment program for novel coronavirus pneumonia (COVID-19)

A prospective randomized double-blind placebo-controlled study of traditional Chinese medicine staging regimen in the treatment of novel coronavirus pneumonia (COVID-19)

A randomized, open-label study to evaluate the efficacy and safety of Lopinavir-Ritonavir in patients with mild novel coronavirus pneumonia (COVID-19)

For peer review only

A randomised, open, controlled trial for darunavir/cobicistat or Lopinavir/ritonavir combined with thymosin a1 in the treatment of novel coronavirus pneumonia (COVID-19)

A randomized controlled trial for the efficacy and safety of Baloxavir Marboxil, Favipiravir tablets in novel coronavirus pneumonia (COVID-19) patients who are still positive on virus detection under the current antiviral therapy

Randomized, open-label, controlled trial for evaluating of the efficacy and safety of Baloxavir Marboxil, Favipiravir, and Lopinavir-Ritonavir in the treatment of novel coronavirus pneumonia (COVID-19) patients

Recommendations of Integrated Traditional Chinese and Western Medicine for Novel Coronavirus Pneumonia (COVID-19)

Therapeutic effect of hydroxychloroquine on novel coronavirus pneumonia (COVID-19)

Safety and efficacy of umbilical cord blood mononuclear cells conditioned medium in the treatment of severe and critically novel coronavirus pneumonia (COVID-19): a randomized controlled trial

A Randomized,Open,Controlled Small Sample Clinical Study to Evaluate the Efficacy and Safety of ASC09/Ritonavir Compound Tablets and Ritonavir for 2019-nCoV Pneumonia

Vitamin C Infusion for the Treatment of Severe 2019-nCoV Infected Pneumonia: a Prospective Randomized Clinical Trial

Safety and efficacy of umbilical cord blood mononuclear cells in the treatment of severe and critically novel coronavirus pneumonia(COVID-19): a randomized controlled clinical trial

A multicenter, randomized, open, controlled clinical study to evaluate the efficacy and safety of recombinant cytokine gene-derived protein injection in combination with standard therapy in patients with novel coronavirus infection

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Severe novel coronavirus pneumonia (COVID-19) patients treated with ruxolitinib in combination with mesenchymal stem cells: a prospective, single blind, randomized controlled clinical trial

A prospective, randomized,open, multi-center clinical trial evaluating the efficiency and safety of Reduning injection in the treatment of novel coronavirus pneumonia (COVID-19)

Community based prevention and control for Chinese medicine in the treatment of novel coronavirus pneumonia (COVID-19) in the isolated population

Clinical study for community based prevention and control strategy of novel coronavirus pneumonia (COVID-19) in the isolate suspected and confirmed population

A Randomized, Open-Label, Multi-Centre Clinical Trial Evaluating and Comparing the Safety and Efficiency of ASC09/Ritonavir and Lopinavir/Ritonavir for Confirmed Cases of Novel Coronavirus Pneumonia (COVID-19)

A randomized, open-label, blank-controlled, multicenter trial for Shuang-Huang-Lian oral solution in the treatment of novel coronavirus pneumonia (COVID-19)

Efficacy and Safety of Hydroxychloroquine for Treatment of COVID-19

Clinical Study for Human Menstrual Blood-derived Stem Cells in the Treatment of Acute Novel Coronavirus Pneumonia (COVID-19)

A Multicenter, Randomized, Controlled trial for Recombinant Super-Compound Interferon (rSIFN-co) in the Treatment of Novel Coronavirus (2019-nCoV) Pneumonia

A randomized, open-label study to evaluate the efficacy and safety of low-dose corticosteroids in hospitalized patients with novel coronavirus pneumonia (COVID-19)

For peer review only

Nasal high-fow preoxygenation assisted fibre-optic bronchoscope intubation in patients with critical novel coronavirus pneumonia (COVID-19): a randomized clinical trial

Efficacy of therapeutic effects of hydroxycholoroquine in novel coronavirus pneumonia (COVID-19) patients(randomized open-label control clinical trial)

Efficacy of Chloroquine and Lopinavir/ Ritonavir in mild/general novel coronavirus (CoVID-19) infections: a prospective, open-label, multicenter randomized controlled clinical study

A randomized, parallel controlled trial for the efficacy and safety of Sodium Aescinate Injection in the treatment of patients with pneumonia (COVID-19)

Effect evaluation and prognosis of Chinese medicine based on Novel Coronavirus Pneumonia (COVID-19)

A randomized, open, parallel-controlled clinical trial on the efficacy and safety of Jingyebaidu granules in treating novel coronavirus pneumonia (COVID-19)

Clinical study of nebulized Xiyanping injection in the treatment of novel coronavirus pneumonia (COVID-19)

Convalescent plasma for the treatment of severe and critical novel coronavirus pneumonia (COVID-19): a prospective randomized controlled trial

A multicenter, randomized, open label, controlled trial for the efficacy and safety of ASC09/ Ritonavir compound tablets and Lopinavir/ Ritonavir (Kaletra) and Arbidol tablets in the treatment of novel coronavirus pneumonia (COVID-19)

A randomized controlled study to evaluate the efficacy and safety of hydroxychloroquine for mild and moderate COVID-19 Infectious diseases

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Multicenter, randomized, open clinical study of Hydroxychloroquine Sulfate tablets in the treatment of common patients with novel coronavirus pneumonia (COVID-19)

A randomized, open-label, controlled trial for the efficacy and safety of hydroxychloroquine sulfate tablets in the treatment of patients with severe novel coronavirus pneumonia (COVID-19)

Study on Prevention and Treatment of Novel Coronavirus Pneumonia (COVID-19) With Traditional Chinese Medicine

A multicenter, randomized controlled trial for the efficacy and safety of tocilizumab in the treatment of new coronavirus pneumonia (COVID-19)

A randomized, open, controlled trial for diammonium glycyrrhizinate enteric-coated capsules combined with vitamin C tablets in the treatment of common novel coronavirus pneumonia (COVID-19) in the basic of clinical standard antiviral treatment to evaluate the safety and efficiency

Babaodan Capsule used for the adjuvant treatment of severe novel coronavirus pneumonia (COVID-19)

A randomized, open-label, blank-controlled, multicenter trial for Polyinosinic-Polycytidylic Acid Injection in the treatment of novel coronavirus pneumonia (COVID-19)

A multicenter, randomized, controlled trial for integrated chinese and western medicine in the treatment of novel coronavirus pneumonia (COVID-19) based on the 'Truncated Torsion' strategy

A multicenter, randomized, open, controlled trial for the efficacy and safety of Shen-Qi Fu-Zheng injection in the treatment of novel coronavirus pneumonia (COVID-19)

A multicenter, randomized, open and controlled trial for the efficacy and safety of Kang-Bing-Du granules in the treatment of novel coronavirus pneumonia (COVID-19)

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Traditional Chinese medicine cooperative therapy for patients with novel coronavirus pneumonia (COVID-19): a randomized controlled trial

Randomized controlled trial for TCM syndrome differentiation treatment impacting quality of life of post-discharge patients with novel coronavirus pneumonia (COVID-19)

Clinical study for the integration of traditional Chinese and western medicine in the treatment of novel coronavirus pneumonia (COVID-19)

Clinical Investigation of Natural Killer Cells Treatment in Pneumonia Patients Infected With 2019 Novel Coronavirus

A prospective, randomized, open-label, controlled clinical study to evaluate the preventive effect of hydroxychloroquine on close contacts after exposure to the Novel Coronavirus Pneumonia (COVID-19)

Optimization of treatment and diagnosis plan for critically ill patients

Clinical Trial for Tanreqing Capsules in the Treatment of Novel Coronavirus Pneumonia (COVID-19)

A randomized controlled trial for honeysuckle decoction in the treatment of patients with novel coronavirus (COVID-19) infection

A randomized, double-blind, parallel, controlled trial for comparison of phosphoric chloroquine combined with standard therapy and standard therapy in serious/critically ill patients with novel coronavirus pneumonia (COVID-19)

Clinical Study of Human Umbilical Cord Mesenchymal Stem Cells in the Treatment of Severe COVID-19

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Evaluating the Efficacy and Safety of Bromhexine Hydrochloride Tablets Combined With Standard Treatment/ Standard Treatment in Patients With Suspected and Mild Novel Coronavirus Pneumonia (COVID-19)

A randomized, double-blind, parallel, controlled trial for comparison of phosphoric chloroquine combined with standard therapy and standard therapy in mild/common patients with novel coronavirus pneumon (COVID-19)ia

Application of Regulating Intestinal Flora in the Treatment of Severe Novel Coronavirus Pneumonia (COVID-19)

A multicenter, randomized controlled trial for the efficacy and safety of Alpha lipoic acid (iv) in the treatment of patients of severe novel coronavirus pneumonia (COVID-19)

A randomized, open-label, controlled clinical trial for azvudine in the treatment of novel coronavirus pneumonia (COVID-19)

A randomized, open and controlled clinical trial for traditional Chinese medicine in the treatment of novel coronavirus pneumonia (COVID-19)

The efficacy and safety of carrimycin treatment in patients with novel coronavirus pneumonia (COVID-19): a multicenter, randomized, open-label, controlled trial

Hydroxychloroquine treating novel coronavirus pneumonia (COVID-19): a randomized controlled, open label, multicenter trial

A multicenter, randomized, controlled trial for integrated Chinese and western medicine in the treatment of ordinary novel coronavirus pneumonia (COVID-19) based on the ' Internal and External Relieving -Truncated Torsion' strategy

Efficacy and Safety of Corticosteroids in COVID-19: A Prospective Randomized Controlled Trails

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Efficacy and Safety of Fuzheng Huayu Tablets in Post-COVID-19 Patients With Pulmonary Inflammation and Fibrosis : A Multicenter Double-blind Randomized Controlled Trial

Evaluation the Efficacy and Safety of Hydroxychloroquine Sulfate in Comparison with Phosphate Chloroquine in Severe Patients with Novel Coronavirus Pneumonia (COVID-19): a Randomized, Open-Label, Parallel, Controlled Trial

Evaluation the Efficacy and Safety of Hydroxychloroquine Sulfate in Comparison with Phosphate Chloroquine in Mild and Commen Patients with Novel Coronavirus Pneumonia (COVID-19): a Randomized, Open-label, Parallel, Controlled Trial

A Single-blind, Randomized, Controlled Clinical Trial for Chloroquine Phosphate in the treatment of Novel Coronavirus Pneumonia 2019 (COVID-19)

A randomized controlled trial for Traditional Chinese Medicine in the treatment for Novel Coronavirus Pneumonia (COVID-19)

A Randomized Controlled Trial for Qingyi No. 4 Compound in the treatment of Convalescence Patients of Novel Coronavirus Pneumonia (COVID-19)

Efficacy and safety of honeysuckle oral liquid in the treatment of novel coronavirus pneumonia (COVID-19): a multicenter, randomized, controlled, open clinical trial

Chinese Medicine Promotes Rehabilitation Recommendations after 2019 Novel Coronavirus Infection (COVID-19)

A randomized controlled trial for the Efficacy of Ultra Short Wave Electrotherapy in the treatment of Novel Coronavirus Pneumonia (COVID-19)

A prospective, multicenter, open-label, randomized, parallel-controlled trial for probiotics to evaluate efficacy and safety in patients infected with 2019 novel coronavirus pneumonia (COVID-19)

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A randomized controlled trial for the efficacy of Dao-Yin in the prevention and controlling novel coronavirus pneumonia (COVID-19)

Clinical Study of Chloroquine Phosphate in the Treatment of Severe Novel Coronavirus Pneumonia (COVID-19)

A randomized controlled Trial for therapeutic efficacy of Recombinant Human Interferon alpha 1b Eye Drops in the treatment of elderly with novel coronavirus pneumonia (COVID-19)

Clinical trials of mesenchymal stem cells for the treatment of pneumonitis caused by novel coronavirus (COVID-19)

A randomized, open-label, controlled trial for the safety and efficiency of Kesuting syrup and Keqing capsule in the treatment of mild and moderate novel coronavirus pneumonia (COVID-19)

An open randomized controlled trial for Chloroquine phosphate and Hydroxychloroquine sulfate in the treatment of severe novel coronavirus pneumonia (COVID-19)

A multicenter, randomized, open-label, parallel controlled trial for the safety and efficacy of Integrated Chinese and Western Medicine in treatment of non-critical novel coronavirus pneumonia (COVID-19): pilot study

Liu-Zi-Jue Qigong and Acupressure Therapy for Pulmonary Function and Quality of Life in Patient with Severe novel coronavirus pneumonia (COVID-19): A Randomized Controlled Trial

The efficacy and safety of Triazavirin for 2019 novel coronary pneumonia (COVID-19): a multicenter, randomized, double blinded, placebo-controlled trial

Clinical study for novel NLRP Inflammasome inhibitor (Tranilast) in the treatment of novel coronavirus (COVID-19)

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Optimization Protocal of Integrated Traditional Chinese and Western Medicine Treatment for Novel Coronavirus Pneumonia (COVID-19)

A randomized controlled trial for the efficacy of ozonated autohemotherapy in the treatment of Novel Coronavirus Pneumonia (COVID-19)

Multicenter randomized controlled trial for rhG-CSF in the treatment of novel coronavirus pneumonia (COVID-19)

A randomized, double-blind, parallel-controlled, trial to evaluate the efficacy and safety of anti-SARS-CoV-2 virus inactivated plasma in the treatment of severe novel coronavirus pneumonia patients (COVID-19)

Efficacy evaluation of inhalation of mycobacterium vaccae injection for treating Novel coronavirus pneumonia (COVID-19) and the mechanism study of prevention and treatment of respiratory virus infection

Adaptive, Randomized, Single-blind, Three-arm Parallel Controlled Clinical Trial of Yinhu Qingwen Decoction in the Treatment of Mild / Common CoVID-19

A randomized, open-label, controlled trial for the efficacy and safety of Farpiravir Tablets in the treatment of patients with novel coronavirus pneumonia (COVID-19)

Effect of early pulmonary training on lung function and quality of life for novel coronavirus pneumonia (COVID-19) patients

A parallel, randomized controlled clinical trial for the efficacy and safety of Pediatric Huatanzhike granules (containing ipecacuanha tincture) in the treatment of mild and moderate novel coronavirus pneumonia (COVID-19)

Clinical comparative study of PD-1 mAb in the treatment of severe and critical patients with novel coronavirus disease (COVID-19)

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A randomized, double-blind, parallel, controlled trial for comparison of phosphoric chloroquine combined with standard therapy and standard therapy in mild/common patients with novel coronavirus pneumonia (COVID-19)

A study for the intervention of Xiangxue antiviral oral solution and Wu-Zhi-Fang-Guan-Fang on close contacts of novel coronavirus pneumonia (COVID-19)

Traditional Chinese Medicine in the treatment of novel coronavirus pneumonia (COVID-19): a multicentre, randomized controlled trial

Shen-Fu injection in the treatment of severe novel coronavirus pneumonia (COVID-19): a multicenter, randomized, open-label, controlled trial

A Randomized, Open Label, Controlled Clinical Study to Evaluate the Recombinant Human Angiotensin-converting Enzyme 2 (rhACE2) in Adult Patients With COVID-19

An open randomized controlled trial for Chloroquine phosphate and Hydroxychloroquine sulfate in the treatment of mild and common novel coronavirus pneumonia (COVID-19)

Multicenter study for the treatment of Dipyridamole with novel coronavirus pneumonia (COVID-19)

A multicenter, randomized, double-blind, controlled clinical trial for leflunomide in the treatment of novel coronavirus pneumonia (COVID-19)

Randomized controlled trial for the efficacy of dihydroartemisinine piperaquine in the treatment of mild/common novel coronavirus pneumonia (COVID-19)

A multicenter study for efficacy of intelligent psychosomatic adjustment system intervention in the treatment of novel coronavirus pneumonia (COVID-19) patients with mild to moderate anxiety and depression

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Umbilical cord Wharton's Jelly derived mesenchymal stem cells in the treatment of severe novel coronavirus pneumonia (COVID-19)

A randomized, open-label, controlled trial for the efficacy and safety of Adalimumab Injection in the treatment of patients with severe novel coronavirus pneumonia (COVID-19)

A multicenter randomized controlled trial for ozone autohemotherapy in the treatment of novel coronavirus pneumonia (COVID-19)

Safety and effectiveness of human umbilical cord mesenchymal stem cells in the treatment of acute respiratory distress syndrome of severe novel coronavirus pneumonia (COVID-19)

A multicenter, randomized, open, parallel controlled trial for the evaluation of the effectiveness and safety of Xiyanping injection in the treatment of common type novel coronavirus pneumonia (COVID-19)

A clinical study for "Huo-Shen" particles in the treatment of novel coronavirus pneumonia (COVID-19)

A randomized controlled trial for high-dose Vitamin C in the treatment of severe and critical novel coronavirus pneumonia (COVID-19) patients

Clinical Trial for Human Mesenchymal Stem Cells in the Treatment of Severe Novel Coronavirus Pneumonia (COVID-19)

Randomized, parallel control, open trial for Qing-Wen Bai-Du-Yin combined with antiviral therapy in the treatment of novel coronavirus pneumonia (COVID-19)

Clinical Trial for Recombinant Human Interleukin-2 in the Treatment of Novel Coronavirus Pneumonia (COVID-19)

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Key techniques of umbilical cord mesenchymal stem cells for the treatment of novel coronavirus pneumonia (COVID-19) and clinical application demonstration

Experimental study of novel coronavirus pneumonia rehabilitation plasma therapy severe novel coronavirus pneumonia (COVID-19)

Efficacy and safety of Lopinavir and Ritonavir in the treatment of novel coronavirus pneumonia (COVID-19)

Clinical Study on Syndrome Differentiation of TCM in Treating Severe and Critical novel coronavirus pneumonia (COVID-19)

A Phase II, Multicenter, Randomized, Double-blind, Placebo-controlled Trial to Evaluate the Efficacy and Safety of Human Umbilical Cord-derived Mesenchymal Stem Cells in the Treatment of Severe COVID-19 Patients

Human Umbilical Cord Mesenchymal Stem Cells Treatment for Pneumonia Patients Infected by 2019 Novel Coronavirus

Effect of pulmonary rehabilitation on pulmonary function, exercise capacity, activity of daily living and quality of life in survivors due to severe or critial novel coronavirus pneumonia (COVID-19)

Study for the efficacy of Kangguan No. 1-3 prescription in the treatment of novel coronavirus pneumonia (COVID19)

Study of Pinavir / Ritonavir Tablets (Trade Name: Kelizhi) Combined with Xiyanping Injection for Novel Coronavirus Pneumonia (COVID-19)

Clinical study of mesenchymal stem cells in treating severe novel coronavirus pneumonia (COVID-19)

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Clinical Reseach of Acupuncture in the treatment of Novel Coronavirus Pneumonia (COVID-19)

the Efficacy and Safety of Favipiravir for novel coronavirus–infected pneumonia: A multicenter, randomized, open, positive, parallel-controlled clinical study

Efficacy and Safety of Jing-Yin Granule in the treatment of novel coronavirus pneumonia (COVID-19) wind-heat syndrome

Evaluation Danoprevir sodium tablets combined with ritonavir in the treatment of novel coronavirus infection (COVID-19): a randomized, open and controlled trial

Clinical study on novel coronavirus pneumonia (COVID-19) patients in Tibetan Plateau promoted by individualized nutritional assessment and supportive treatment

A study for the key technology of mesenchymal stem cells exosomes atomization in the treatment of novel coronavirus pneumonia (COVID-19)

Clinical research program of continuous renal replacement therapy with adsorption filter for the treatment of the novel coronavirus pneumonia (COVID-19)

Study on Prevention and Treatment of Novel Coronavirus Pneumonia (COVID-19) With Traditional Chinese Medicine

A Randomized, Open-label Study to Evaluate the Efficacy and Safety of Pirfenidone in Patients With Severe and Critical Novel Coronavirus Infection

A prospective randomized controlled trial for a Self-made Gastroscope isolation mask for preventing and controlling the novel coronavirus pneumonia (COVID-19) Epidemic period

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Clinical application of inhaled acetylcysteine solution in the treatment of novel coronavirus pneumonia (COVID-19)

Clinical trial for umbilical cord blood CIK and NK cells in the treatment of mild and general patients infected with novel coronavirus pneumonia (COVID-19)

A Phase IIb Randomized Placebo-Controlled Study to Examine the Efficacy and Safety of DAS181 for the Treatment of Severe Influenza Infection

A randomized, open-label, controlled and single-center trial to evaluate the efficacy and safety of anti-SARS-CoV-2 inactivated convalescent plasma in the treatment of novel coronavirus pneumonia (COVID-19) patient

Efficacy and safety of Xue-Bi-Jing injection in the treatment of severe cases of novel coronavirus pneumonia (COVID-19)

A randomized, double-blind, placebo-controlled trial for evaluation of the efficacy and safety of bismuth potassium citrate capsules in the treatment of patients with novel coronavirus pneumonia (COVID-19)

Efficacy and safety of chloroquine phosphate inhalation combined with standard therapy in the treatment of novel coronavirus pneumonia (COVID-19)

Application of rehabilitation lung exercise eight-segment exercise in patients with novel coronavirus pneumonia (COVID-19)

A Clinical Trial Study for the Influence of TCM Psychotherapy on Negative Emotion of Patients with Novel Coronavirus Pneumonia (COVID-19) Based on Network Platform

Application of rehabilitation and Lung eight-segment exercise in front-line nurses in the prevention of novel coronavirus pneumonia (COVID-19) epidemic

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Influence of nasal high-fow preoxygenation on video laryngoscope for emergency intubation in patients with critical novel coronavirus pneumonia (COVID-19)

A Randomized Controlled Trial for the Influence of TCM Psychotherapy on Negative Emotion of Patients with Novel Coronavirus Pneumonia (COVID-19) Based on Network Platform

A randomized parallel controlled trial for LIUSHENWAN in Treatment of Novel Coronavirus Pneumonia (COVID-19)

Efficacy and safety of lipoic acid injection in reducing the risk of progression in common patients with novel coronavirus pneumonia (COVID-19)

Study for meditation assists for the rehabilitation of patients with novel coronaviruse pneumonia (COVID-19)

Positively Controlled Clinical Study of Compound Yuxingcao Mixture in the Treatment of the Novel Coronavirus Pneumonia (COVID-19)

Study for the Effectiveness and Safety of Yi-Qi Hua-shi Jie-Du-Fang in the Treatment of the Novel Coronavirus Pneumonia (COVID-19)

Randomized, open, blank controlled trial for the efficacy and safety of recombinant human interferon alpha 1beta in the treatment of Wuhan patients with novel coronavirus pneumonia (COVID-19)

The clinical value of corticosteroid therapy timing in the treatment of novel coronavirus pneumonia (COVID-19): a prospective randomized controlled trial

A multicentre, randomized, controlled trial for the Ba-Duan-Jin in the adjunctive treatment of patients with common type novel coronavirus pneumonia (COVID-19)

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HUMSCs and Exosomes Treating Patients with Lung Injury following Novel Coronavirus Pneumonia (COVID-19)

Clinical Study of Tetrandrine Tablets Adjuvant Treatment With COVID-19

Clinical Study of NK Cells in the Treatment of Novel Coronavirus Pneumonia (COVID-19)

Zedoary Turmeric Oil for Injection in the treatment of Novel Coronavirus Pneumonia (COVID-19): a randomized, open, controlled trial

Efficacy and safety of Ma-Xing-Gan-Shi decoction in the treatment of novel coronavirus pneumonia (COVID-19)

Multi-Center Clinical Study on the Treatment of Patients with Novel Coronavirus Pneumonia (COVID-19) by Ebastine

Research for the mechanism of improvement of novel coronavirus pneumonia (COVID-19) patients' pulmonary exudation by continuous renal replacement therapy

Efficacy and safety of honeysuckle oral liquid in the treatment of novel coronavirus pneumonia (COVID-19): a multicenter, randomized, controlled, open clinical trial

Clinical Prediction and Intervention of Pulmonary Function Impairment in Patients with Novel Coronavirus Pneumonia (COVID-19)

Tozumab combined with adamumab(Qletli) in the treatment of severe and critically ill patients with novel coronavirus pneumonia (COVID-19):A prospective, single-center, randomized, parallel controlled trial

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Study on the application of convalescent plasma therapy in severe COVID-19

Favipiravir Combined With Tocilizumab in the Treatment of Corona Virus Disease 2019-A Multicenter, Randomized and Controlled Clinical Trial Study

An anaesthesia procedure and extubation strategy for reducing patient agitation and cough after extubation that can be used to prevent the spread of SARS-CoV-2 and other infectious viruses in the operating Room

The Efficacy and Safety of Bevacizumab in Severe or Critical Patients With COVID-19--a Multicenter Randomized Controlled Clinical Trial

An evaluative clinical study: efficacy and safety of of Prolongin (Enoxaparin Sodium Injection) in treatment of hospitalized adult patients with common novel coronavirus pneumonia (COVID-19)

A randomized, parallel controlled open-label trial to evaluate the efficacy and safety of Prolongin (Enoxaparin Sodium Injection) in adult hospitalized patients with novel coronavirus pneumonia (COVID-19)

Convalescent plasma for the treatment of common COVID-19: a prospective randomized controlled trial

A randomized, blinded, controlled, multicenter clinical trial to evaluate the efficacy and safety of Ixekizumab combined with conventional antiviral drugs in patients with novel coronavirus pneumonia (COVID-19)

Observation Of Clinical Efficacy And Safety Of Bufonis Venenum Injection In The Treatment Of Severe Novel Coronavirus Pneumonia (COVID-19)

Clinical Study of Chloroquine Phosphate in the Treatment of 2019-nCoV Infection

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Prognosis Investigation and Intervention Study on Patients with novel coronavirus pneumonia (COVID-19) in recovery period Based on Community Health Management

A clinical study for effectiveness and safety evaluation for recombinant chimeric COVID-19 epitope DC vaccine in the treatment of novel coronavirus pneumonia

Study for the therapeutic effect and mechanism of traditional Chinese medicine in the treatment of novel coronavirus pneumonia (COVID-19)

A randomized, open-label, controlled trial for the efficacy and safety of Ulinastatin Injection in the treatment of patients with severe novel coronavirus pneumonia (COVID-19)

Exocarpium Citri Grandis Relieves Symptoms of Novel Coronavirus Pneunomia (COVID-19): a Randomized Controlled Clinical Trial

An Adaptive, Randomized, Double-blind, Parallel-controlled Clinical Trial of Yinhu Qingwen Granula for the Treatment of Severe CoVID-19

Efficacy and Safety of Pirfenidone in the Treatment of Severe Post-Novel Coronavirus Pneumonia (COVID-19)) Fibrosis:a prospective exploratory experimental medical study

Evaluation of the effect of taking Newgen beta-gluten probiotic composite powder to nutrition intervention of patients with novel coronavirus pneumonia (COVID-19)

Evaluation of the effect of taking tricholoma matsutake, cannabis sativa capsule and dendrobium candidum to nutrition intervention of patients with novel coronavirus pneumonia (COVID-19) during convalescence.

Prospective, open-label, controlled, multicenter cohort study of long-acting interferon plus ribavirin in patients with COVID-19 infection

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The treatment and diagnosis plan of integrated traditional Chinese and Western medicine for novel coronavirus pneumonia (COVID-19)

A randomized, double-blind, parallel-controlled trial to evaluate the efficacy and safety of anti-SARS-CoV-2 virus inactivated plasma in the treatment of severe novel coronavirus pneumonia (COVID-19)

Effectiveness of "Liu-Zi-Jue" combined with respiratory muscle training for respiratory function in COVID-19 patients: a demonstration study for the key techniques

Gu-Shen Ding-Chuan-Wan used in the recovery phase of 2019-nCoV pneumonia(FeiPiQiXuZheng): a randomized, open-label, controlled clinical trial

An open, multi-center, control, exploratory clinical study of human NK cells and UC-MSCs transplantation for severe novel coronavirus pneumonia

Clinical Trial of Favipiravir Tablets Combine With Chloroquine Phosphate in the Treatment of Novel Coronavirus Pneumonia

Efficacy and safety of Chinese Herb Medicine Hua-Shi Bai-Du granules in patients with novel coronavirus pneumonia (COVID-19) in Wuhan, China: a prospective, randomized, controlled, open-label trial

Effect of Auricular point pressing on insomnia of 2019 new coronavirus pneumonia patients: a randomized controlled trial.

Psychological Support for Diagnosed Patients with novel coronavirus pneumonia (COVID-19)

Prospective exploratory clinical research on the efficacy and safety of pirfenidone capsules in the treatment of severe novel coronavirus pneumonia (COVID-19)

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Effectiveness and safety of hydroxychloroquine sulfate in the preventive treatment of novel coronavirus pneumonia (COVID-19)

Safety and Efficacy of Intravenous Infusion of Bone Marrow-Derived Mesenchymal Stem Cells in Severe Patients With Coronavirus Disease 2019 (COVID-19): A Phase 1/2 Randomized Controlled Trial

A multicenter, single-blind, randomized controlled clinical trial for chloroquine phosphate in the treatment of 2019 novel coronavirus-infected pneumonia

A Multi-center, Randomized, Parallel-Controlled Clinical Trial of the Application of A Hydrogen-Oxygen Generator With Nebulizer in the Improvement of Symptoms in Patients Infected With COVID-19

A Phase I/II Study of Universal Off-the-shelf NKG2D-ACE2 CAR-NK Cells Secreting IL15 Superagonist and GM-CSF-neutralizing scFv for Therapy of COVID-19

Safety and Effectiveness of Mesenchymal Stem Cells in the Treatment of Pneumonia of Coronavirus Disease 2019

Metabolic abnormalities and rearrangement reversal treatment of patient with novel coronavirus pneumonia (COVID-19)

The protective effect of sleep psychology and music therapy in patients with new coronary pneumonia (mild and moderate type)

Safety and Efficacy Study of Allogeneic Human Dental Pulp Mesenchymal Stem Cells to Treat Severe Pneumonia of COVID-19:a Single-center, Prospective, Randomised Clinical Trial

Efficacy and safety of Nintedanib esilate soft capsules in the treatment of pulmonary fibrosis in healed moderate to severe patients of novel coronavirus pneumonia (COVID-19): a single-center, randomized, placebo-controlled trial

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Clinical study for prevention and treatment of digestive tract lesions caused by novel coronavirus pneumonia (COVID-19)

The Mechanism, Clinical Outcome and Therapeutic Intervention of Corona Virus Disease 2019 Patients Whose Nucleic Acids Changed From Negative to Positive

Efficacy and Safety of Anluohuaxian in the Treatment of Rehabilitation Patients With Corona Virus Disease 2019-A Multicenter, Open, Randomized Controlled Study

Application of non-invasive multichannel cardiopulmonary monitoring cloud management platform in patients with Novel Coronavirus Pneumonia (COVID-19)

Development and application of TCM body regulating protection scheme for the convalescent population of novel coronavirus pneumonia (COVID-19)

A Randomized, Double-blind, Placebo-controlled Phase II Clinical Trial to Evaluate the Safety and Immunogenicity of the Recombinant Novel Coronavirus Vaccine (Adenovirus Vector) in Healthy Adults Aged Above 18 Years

Clinical study for natural killer (NK) cells from umbilical cord blood in the treatment of viral pneumonia include novel coronavirus pneumonia (COVID-19)

Evaluation of the safety and immunogenicity of inactivated Novel Coronavirus Pneumonia (COVID-19) vaccine (Vero cells) in healthy population aged 6 years and above: a randomized, double-blind, placebo parallel-controlled phase I/II clinical trial

A clinical study for the effect of remote monitoring exercise rehabilitation on the discharged patients with novel coronavirus pneumonia (COVID-19)

Clinical Research on Treatment of Novel Coronavirus Pneumonia with "Guangdong Pneumonia NO.1"

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A Randomized, Double-Blinded, Placebo-Controlled, Phase I/II Clinical Trial, to Evaluate the Safety and Immunogenicity of the SARS-CoV-2 Inactivated Vaccine in Healthy Adults Aged 18~59 Years

Efficacy of Chinese Herbal Tea in the Prevention of Novel Coronavirus Pneumonia (COVID-19) for Returning Workers in Wuhan: a Randomized Controlled Trial

Study on the prevention of COVID-19 with Chinese medicine regulating constitution

Effecacy and Safety of Hyperbaric Oxygen Therapy to patients with novel coronavirus pneumonia (COVID-19)

Efficacy and Safety of ulinastatin in the Treatment of novel coronavirus pneumonia (COVID-19)

Moxibustion Plus Cupping in Convalescent Patients With COVID-19: A Randomized Clinical Trial

A multicenter, randomized, double-blind, parallel-controlled trial for Qi-Mai-Fei-Luo-Ping Mixture in the improvement of lung function of novel coronavirus pneumonia (COVID-19) in the convalescent period

A multicenter randomized, double-blind, placebo-controlled trial for Sheng-Mai-Yin for improvement of the pulmonary heart function related symptoms of convalescence patients of new coronavirus pneumonia

A randomized, double-blind, placebo-controlled, multicentre clinical study on the improvement of novel coronavirus pneumonia (COVID-19) convalescent sleep mood disorder with shumian capsule

A multicenter, randomized, open, controlled trial for the efficacy and safety of oral kolimycin in the treatment of patients with new coronavirus pneumonia (CoVID-19)

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Protective Effect of Aspirin on COVID-19 Patients

Study for efficacy and safety of Jie-Xing-Jun-Zi granules in the Treatment of convalescent patients of novel coronavious pneumonia (COVID-19)

A randomized clinical trial for the efficacy and safety of Aliskiren and Nifedipine in novel coronavirus pneumonia (COVID-19) patients with hypertension

Rqesearch on the Application of Traditional Chinese Medicine Traditional Techniques to Prevent and Control New Coronary Pneumonia

Xiaoyao capsule improves COVID-19 convalescence sleep mood disorder in a randomized, double-blind, placebo-controlled, multicenter clinical study

Application of pulmonary rehabilitation exercise in the treatment of novel coronavirus pneumonia (COVID-19) patients during convalescence period.

Evaluation of the safety and immunogenicity of inactivated novel coronavirus (2019-CoV) vaccine (Vero cells) in healthy population aged 3 years and above: a randomized, double-blind, placebo parallel-controlled phase I/II clinical trial

Study for using sulfate in the prevention and control of novel coronavirus pneumonia (COVID-19) in high and low prevalence communities

A randomized, double-blind, controlled trial for Bufei Huoxue Capsule in the treatment of convalescent COVID-19 patients

A randomized, double-blind, placebo, parallel controlled trial for Pien-Tze-Huang Capsules in treating convalescent novel coronavirus pneumonia (COVID-19) patient with SARS-CoV-2 positive.

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Clinical trial for the washed microbiota transplantation in the treatment of novel coronavirus pneumonia (COVID-19) patients suspected with gut microbiota dysbiosis

A randomized,double blinded, parallel controlled clinical trial for azvudine in the treatment of novel coronavirus pneumonia (COVID-19)

A Randomized, Double-blind, Placebo-controlled Clinical Trial, to Evaluate Safety and Immunogenicity of Inactivated SARS-CoV-2 Vaccine (Vero Cell), in Healthy Elderly Aged 60 Years and Above

The Effect of Berberine on Intestinal Function and Inflammatory Mediators in Severe Patients With Covid-19

A randomized controlled trial for the efficacy and safety of artemisinin-pipequine tablets in the treatment of the mild and common type novel coronavirus pneumonia (COVID-19) patients whose nCoV Nucleic acid did not turn negative after treated by hydroxychloroquine and Abidor

A multicenter, randomized, double-blind, placebo-controlled trial for Lu-Dang-Shen oral liquid in the treatment of reduced digestion function of convalescent new coronavirus pneumonia (COVID-19) patient

A randomized controlled trial for determination of the clinical effect of Internet-assisted pulmonary rehabilitation on patients with novel coronavirus pneumonia (COVID-19)

A randomized, open-label, blank-controlled, multicenter trial for Shuang-Huang-Lian oral solution in the treatment of novel coronavirus pneumonia (COVID-19)

A Randomized, Double-blind, Placebo-controlled, Phase Ia/IIa Trial of an Inactivated SARS-CoV-2 Vaccine in Healthy People Aged 18 to 59 Years

Usefulness of abdominal massage and ST-36 (Zusanli) acupressure in improving appetite of novel coronavirus pneumonia (COVID-19) patients treated in makeshift hospital

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Study for the improvement of the quality of life of severe patients of novel coronavirus pneumonia (COVID-19) by the integrated Chinese medicine and Western medicine therapy

A Randomized, Double-blind, Placebo-controlled trial for determination of the Efficacy and Safety of Baicalein in Hospitalized Adult Mild and Moderate Patients of novel coronavirus pneumonia (COVID-19)

Rqesearch on the Application of Traditional Chinese Medicine Traditional Techniques to Prevent and Control New Coronary Pneumonia

Study for Traditional Chinese Medicine in the treatment of novel coronavirus pneumonia (COVID-19)

A Randomized, Double-blind, Placebo-controlled, Phase I Clinical Study to Evaluate the Tolerability, Safety, Pharmacokinetic Profile and Immunogenicity of JS016 (Anti-SARS-CoV-2 Monoclonal Antibody) Injection in Chinese Healthy Subjects After Intravenous Infusion of Single Dose

Evaluating the Efficacy and Safety of Bromhexine Hydrochloride Tablets in Treating Pediatric novel coronavirus pneumonia (COVID-19)

A Multi-center, Double-blind, Randomized, Placebo Parallel Controlled, Safety and Tolerability Phase I Clinical Trial of Recombinant Novel Coronavirus Vaccine (CHO Cells) in Healthy People Between 18 and 59 Years of Age

The effect of cognitive behavioural therapy on depression, anxiety and stress in patients with COVID-19: a randomized controlled trial

A Phase I clinical trial to evaluate the safety, tolerance and preliminary immunogenicity of different doses of a SARS-CoV-2 mRNA vaccine in population aged 18-59 years and 60 years and above

A Randomized, Blinded, Placebo-controlled Trial to Evaluate the Immunogenicity and Safety of a Recombinant New Coronavirus Vaccine (CHO Cell) With Different Doses and Different Immunization Procedures in Healthy People Aged 18 to 59 Years

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A Randomized, Double-blind, Placebo-controlled, Phase Ib/IIb Trial of an Inactivated SARS-CoV-2 Vaccine in Healthy People Aged ⩾60 Years

A Phase 1 Randomized, Single-blind, Placebo-controlled, Single Ascending Dose Escalation Study of the Safety, Tolerability, and Pharmacokinetics of Human Monoclonal Antibody, BRII-196 Administered Intravenously to Healthy Adult Volunteers

A Phase 1 Randomized, Single-blind, Placebo-controlled, Single Ascending Dose Escalation Study of the Safety, Tolerability, and Pharmacokinetics of Human Monoclonal Antibody, BRII-198 Administered Intravenously to Healthy Adult Volunteers

A Randomized, Double-blinded, Placebo-controlled, Single Ascending Dose, Phase I Study to Evaluate the Tolerability, Safety, Pharmacokinetics of SCTA01 in Healthy Subjects

Sancai granule improves lung and kidney function for patient with COVID-19 pneumonia in the recovery period: a randomized, parallel controlled trial

Clinical study of safety, tolerance, and pharmacokinetics of nebulizer (INF-K + anti-inflammatory factor TFF2) in healthy volunteers and exploration of PK/PD fitting modeling in clinical trials of Novel Coronavirus Pneumonia (COVID-19) patients

A Randomized, Double-blind, Placebo-controlled Phase I Clinical Trial to Evaluate the Safety and Tolerability of Recombinant New Coronavirus Vaccines (CHO Cells) in Healthy People Aged 60 Years and Above

Safety and Immunogenicity of SARS-CoV-2 mRNA Vaccine (BNT162b1) in Chinese Healthy Subjects: A Phase I, Randomized, Placebo-controlled, Observer-blind Study

A Phase I Clinical Study to Evaluate the Safety, Tolerability, Pharmacokinetic Characteristics and Immunogenicity of a Single Dose of MW33 Injection in Healthy Subjects

Safety, Tolerability and Immunogenicity of a Recombinant SARS-CoV-2 Vaccine (Sf9 Cell) in Chinese Healthy Population Aged 18 Years and Older: A Phase I, Single-center, Randomized, Placebo-controlled, Double-blind Study

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Longitudinal research project of Wuhan novel coronavirus pneumonia (COVID-19) patients and important rehabilitation factors

A Phase I Clinical Trial of Influenza virus Vector COVID-19 Vaccine for intranasal Spray (DelNS1-2019-nCoV-RBD-OPT1)

A Single-center, Randomized, Double-blind, Placebo-controlled, Dose Ranging Phase I Study to Evaluate the Safety, Tolerability, and Immunogenicity of INO-4800, Administered Intradermally Followed by Electroporation in Healthy Volunteers

A Randomized, Double-Blinded, Placebo-Controlled, Phase I/II Clinical Trial, to Evaluate the Safety and Immunogenicity of the SARS-CoV-2 Inactivated Vaccine (Vero Cell) in Healthy Population Aged 3-17 Years

Study on Screening and prevention of osteonecrosis of the femoral head during novel coronavirus pneumonia (COVID-19) recovery

Study on TCM intervention programs for novel coronavirus pneumonia (COVID-19) Convalescence and Sequelae based on "COVID-19 case Registration System"

A Randomized, Double-blind, Placebo -Controlled Phase IIb Clinical Trial to Evaluate the Safety and Immunogenicity of Ad5-nCoV in Person 6 Years of Age and Older and Those Who Have Previously Been Vaccinated With Ad5-EBOV

A Randomized, Double-blind, Placebo-controlled, Phase Ib/II Clinical Study to Evaluate the Preliminary Efficacy, Safety, Pharmacokinetic Profiles and Immunogenicity of JS016 in Participants With Mild and Moderate COVID-19 or of SARS-CoV-2 Asymptomatic Infection

Study on traditional Chinese Medicine intervention Program of COVID-19 's convalescence and sequelae based on COVID-19 case Registration system

Evaluation of the safety and immunogenicity of inactivated SARS-CoV-2 Vaccine(Vero Cells) in healthy population aged 18 years and above: a randomized, double-blind, placebo parallel-controlled phase I clinical trial

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A Clinical Trial to Evaluate the Efficacy and Safety of Hydrogen-oxygen Mixed Gas Inhalation in Convalescent Severe/Critically Ill Patients With Novel Coronavirus Pneumonia (COVID-19)

A Phase Ib clinical trial to evaluate the safety, tolerance and preliminary immunogenicity of different doses of a SARS-CoV-2 mRNA vaccine in population aged 18-59 years

The Effects of Narrative Exposure Therapy on COVID-19 Patients with Post-Traumatic Stress Disorder: a Randomized Controlled Trial

Evaluation of the safety and immunogenicity of inactivated SARS-CoV-2 Vaccine (Vero Cells) in healthy population aged 18 years and above: a randomized, double-blind, placebo parallel-controlled phase II clinical trial

A Randomized, Double-Blinded Clinical Trial, to Evaluate the Non-inferiority of the Commercial Scale Inactivated SARS-CoV-2 Vaccine Against That of the Pilot Scale Among Adults Aged 26-45 Years, and the Open-labelled, Bridging Non-inferiority of the Vaccine Induced Immunogenicity in Elderly Against That in Adults

A Phase II Clinical Trial of Influenza virus Vector COVID-19 Vaccine for intranasal Spray (DelNS1-2019-nCoV-RBD-OPT1)

A Multicenter, Randomized, Double-Blind, Placebo-Controlled Phase 2 Clinical Trial of MW33 Injection to Evaluate the Efficacy and Safety in Patients With Mild or Moderate COVID-19

Safety and Immunogenicity of SARS-CoV-2 mRNA Vaccine (BNT162b2) in Chinese Healthy Population: A Phase II, Randomized, Placebo-controlled, Observer-blinded Study

Safety and Immunogenicity of a Recombinant COVID-19 Vaccine (CHO Cell) in Healthy Population Aged 18 Years and Older: A Phase I Study

A Single-center, Randomized, Double-Blinded, Placebo-Controlled, Phase II Clinical Trial of Recombinant COVID-19 Vaccine (Sf9 Cells), in the Subjects From Healthy Aged 18 Years and Above

For peer review only

COVID-19 DNA vaccine phase II clinical trial

A randomized, double-blind, placebo-controlled phase III clinical trial of the effectiveness and safety of inoculation of recombinant new coronavirus vaccine (CHO cells) in the prevention of COVID-19 in people 18 years and older

Efficacy and Safety of High-dose Vitamin C Combined With Traditional Chinese Medicine in the Treatment of Moderate and Severe Coronavirus Pneumonia (COVID-19)

A clinical study on the treatment of severe COVID-19 patients with a new aerosol inhalation agent

A Phase 1 Randomized, Single-blind, Placebo-controlled, Single Ascending Dose Escalation Study of the Safety, Tolerability, and Pharmacokinetics of Human Monoclonal Antibodies, Brii-196 and BRII-198, Administered Intravenously to Healthy Adult Volunteers

A randomized, controlled, multicenter study on the safety and immunogenicity of SARS-CoV-2 inactivated vaccine

A randomized, controlled, multicenter study on the safety and immunogenicity of SARS-CoV-2 inactivated vaccine

A randomized, controlled, multicenter study on the safety and immunogenicity of SARS-CoV-2 inactivated vaccine

A Phase II clinical trial to evaluate the immunogenicity and safety of different doses of a novel coronavirus pneumonia (COVID-19) mRNA vaccine in population aged 18-59 years

A randomized, open-label, blank-controlled, multi-center clinical study for Lian-Hua Qing-Ke Tablets in the treatment of severe novel coronavirus pneumonia

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A randomized, open-label, blank-controlled, multi-center clinical study for Lian-Hua Qing-Ke Tablets in the treatment of COVID-19 patients with mild and common-type

A randomized, open-label, blank-controlled, multi-center clinical study for Lian-Hua Qing-Wen Capsule/Granule in the treatment of asymptomatic patients with novel coronavirus pneumonia

Phase I clinical study to evaluate the safety, tolerability, pharmacokinetic profile and immunogenicity of JMB-2002 injection in Chinese healthy subjects after intravenous infusion of single dose

A Single-center, Randomized, Double-Blinded, Placebo-Controlled, Phase IIb Clinical Trial of Recombinant COVID-19 Vaccine (Sf9 Cells), in the Subjects From Healthy Aged 18 to 85 Years Old

Qing-Jin Yi-Qi granule in the intervention of patients in rehabilitation phase of COVID-19

Real-world study on the prevention effect of Lian-Hua Qing-Wen Capsules on close contacts of new coronavirus pneumonia

A randomized, double-blind, placebo-controlled trial of Shugan Jieyu Capsules for improving sleep and mood disorders during recovery from novel coronavirus pneumonia

A Phase 2 Study to Evaluate the Safety and Efficacy of Human Monoclonal Antibodies, BRII-196 and BRII-198, Administered by Intravenous Infusion for the Treatment of COVID-19 Patients

Comprehensive evaluations of dynamic changes in the immune system of healthy volunteers after vaccination with inactivated Novel Coronavirus Pneumonia (COVID-19) vaccine (Vero cells) (SARS-CoV-2 Vaccine (Vero Cell), Inactivated)

A Study to Evaluate the Safety and Immunogenicity of Concomitant Administration of the SARS-CoV-2 Inactivated Vaccine (Vero Cell) With Quadrivalent Influenza Vaccine in Adults Aged From 18 to 59 Years

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A Single-center, Randomized, Double Blinded, Placebo Controlled, Phase 2 Clinical Trial of Recombinant COVID-19 Vaccine (CHO Cells), in the Subjects From Healthy Aged 18 Years and Above

Clinical study on the treatment of COVID-19 by Qingfei Paidu Granule

Safety and Immunogenicity of a Heterologous Immunization of Recombinant COVID-19 Vaccine (Ad5 Vector) and RBD-based Protein Subunit Vaccine Against COVID-19 in Chinese Healthy Population

A Randomized, Double-blind, Placebo-controlled Phase II Clinical Trial to Evaluate the Immunogenicity and Safety of Recombinant SARS-CoV-2 Fusion Protein Vaccine (V-01) in Healthy Subjects

A Randomized, Double-blind, Placebo-controlled Phase I Clinical Trial to Evaluate the Safety and Immunogenicity of Recombinant SARS-CoV-2 Fusion Protein Vaccine (V-01) in Healthy Subjects

The Impact of Timing of Vaccination on the Immune Reaction to Vaccine against SARS-CoV-2

A Randomized, Double-blind, Placebo-Controlled Phase I/II Clinical Trial to Evaluate the Safety and Immunogenicity of Ad5-nCoV for Inhalation in Adults 18 Years of Age and Older

Phase I/II Clinical Trial to Evaluate the Safety, Tolerability and Immunogenicity of Recombinant SARS-CoV-2 Vaccine (CHO Cell) in Healthy People Aged 3 Years and Older

A Immunobridging and Immunization Schedules Study of COVID-19 Vaccine (Vero Cell), Inactivated in Population Aged = 60 Years Old and 3-17 Years Old Compared With That in Population Aged 18-59 Years Old

Randomized, double-blind, placebo-controlled phase I clinical trial to evaluate the safety and immunogenicity of mRNACOVID-19 vaccine in healthy susceptible populations aged 18 years and older people

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A Randomized, Double-Blinded, Placebo-Controlled, Phase IIb Clinical Trial, to Evaluate the Safety of the Inactivated SARS-CoV-2 Vaccine (Vero Cell) in Healthy Population Aged From 3 to 17 Years.

A Randomized, Controlled, Multicenter Phase 4 Clinic Trial to Evaluate the Immunogenicity and Safety of Combined Immunization of Inactivated SARS-CoV-2 Vaccines (Vero Cell) (COVIV) and 23-valent Pneumococcal Polysaccharide Vaccine (PPV23) or Quadrivalent Inactivated Influenza Vaccine (IIV4)

A Randomized, Controlled, Multi-center Clinical Study To Evaluate The Immunogenicity And Safety Of Combined Immunization of Inactivated SARS-CoV-2 Vaccines (Vero Cell) (COVIV) and 23-valent Pneumococcal Polysaccharide Vaccine (PPV23) or Quadrivalent Inactivated Influenza Vaccine (IIV4)

A Randomized Controlled Trial for Qing Jin Yi Qi granule in the treatment of Convalescence Patients of Novel Coronavirus Pneumonia(COVID-19)

Efficiency and Safety of Nicotinamide-based Supportive Therapy in Lymphopenia for Patients With COVID-19: A Randomized Clinical Trial

To Evaluate the Immunogenicity Bridging Between Different Manufacture Scales and Between Different Lots of Novel Coronavirus Vaccine (Adenovirus Type 5 Vector) (Ad5-nCoV) in Population 6-59 Years of Age.

Safety and Immunogenicity of Sequential Immunization of Inactivated SARS-CoV-2 Vaccine and Recombinant SARS-CoV-2 Vaccine (Ad5 Vector) in Chinese Healthy Adults Aged 18-59 Years: a Randomized, Observer-blind, Parallel-controlled Clinical Trial

An extended Clinical Trial of Influenza virus Vector COVID-19 Vaccine for intranasal Spray (DelNS1-2019-nCoV-RBD-OPT1)

Comprehensive evaluations of dynamic changes in the immune system of healthy volunteers with sequential vaccination of inactivated Novel Coronavirus Pneumonia vaccine (Vero cells) followed by Novel Coronavirus Pneumonia mRNA vaccine (DF104B1)

A randomized, double-blind, placebo-controlled trial for Xinguan Fuzheng Zhuanyin Decoction to reduce the viral load of the delta variant of COVID-19

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A Multi-center, Double-blind, Randomized, Controlled, Phase 2 Study to Evaluate the Immunogenicity and Safety of CpG 1018/Alum-adjuvanted Recombinant SARS-CoV-2 Trimeric S-protein Subunit Vaccine (SCB-2019) in Individuals Aged 18 Years and Older in China

A Phase I, Single Ascending Dose Study to Evaluate the Safety, Tolerability, Pharmacokinetics, and Immunogenicity of LY-CovMab Injection in Chinese Healthy Subjects

Immunogenicity and Safety of an Inactivated COVID-19 Vaccine for Prevention of COVID-19 in Healthy Population Aged 18 Years and Older

A Randomized, Blinded, Placebo-controlled Phase I Clinical Trial of Recombinant New Coronavirus Vaccine (CHO Cells) in the Safety and Tolerability of Healthy People Aged 3 to 17 Years of Age

A Randomized, Double-blinded, Placebo-controlled Phase I Clinical Trial to Evaluate the Safety, Tolerability, and Preliminary Immunogenicity of the Recombinant SARS-CoV-2 Vaccine (CHO Cell) in Healthy Adults Aged 18 Years and Above

A Randomized, Double-blinded, Placebo-controlled Phase II Clinical Trial to Evaluate the Immunogenicity and Safety of the Recombinant SARS-CoV-2 Vaccine (CHO Cell) in Healthy Adults Aged 18 Years and Above

A Randomized and Controlled IV Clinical Trial to Evaluate the Immunogenicity and Safety of COVID-19 Vaccine (Vero Cell), Inactivated Co-administration With Quadrivalent Influenza Vaccine and 23-valent Pneumococcal Polysaccharide Vaccine

A Randomized, Double-blind, Placebo-controlled Phase II Clinical Study to Evaluate the Safety and Immunogenicity of Inactivated SARS-CoV-2 Vaccine (Vero Cells) in Healthy Population Aged 3 to 17 Years

A Randomized, Double-blind, Placebo-controlled Phase I Clinical Study to Evaluate the Safety and Immunogenicity of SARS-CoV-2 Vaccine (Vero Cells), Inactivated in Healthy Population Aged 3 to 17 Years

A Double-blind, Randomized Clinical Trial to Evaluate the Lot-to-lot Consistency, Immunogenicity and Safety of an Inactivated SARS-CoV-2 Vaccine (CoronaVac) in Health Adults Aged 26-45 Years

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Single-center, Randomized, Double-blind, Placebo-controlled Phase I/II Clinical Trial to Evaluate the Safety, Tolerability and Immunogenicity of Recombinant COVID-19 Vaccine (Sf9 Cells) in Healthy People Aged 6-17 Years

A randomized, double-blind, placebo-controlled phase I clinical trial to evaluate the safety and immunogenicity of the Novel Coronavirus inactivated vaccine (Vero cells) in healthy individuals 18 years of age and older

Study on Chinese Medicine Rehabilitation Promotion Program for COVID-19 Patients during Convalescence

Treatment Based on Syndrome Differentiation for 'Three formulations' in Delta novel coronavirus pneumonia (COVID-19): A Multicentre, Open-Label, Randomised Controlled Clinical Trial

Evaluation of clinical efficacy of Chinese medicine in promoting nucleic acid negative transformation of novel coronavirusDelta mutant strain

Study on Prevention and Treatment of Novel Coronavirus Pneumonia (COVID-19) With Traditional Chinese Medicine

A Phase II Clinical Trial to Evaluate the Immunogenicity and Safety of the Recombinant SARS-CoV-2 Vaccine (Chimpanzee Adenoviral Vector, ChAdTS-S) in Participants Aged 18 to 59 Years as well as 60 Years and Older

A Phase I/II Clinical Trial to Evaluate the Immunogenicity and Safety of the SARS-CoV-2 mRNA Vaccine in Healthy Population Aged 60 Years and Older

A Pragmatic, Randomized Clinical Trial of Coronavirus (COVID-19) Vaccinations in Uveitis

Phase I clinical trial to evaluate the safety, tolerability and preliminary immunogenicity of the new coronavirus mRNA vaccine (LVRNA009) in Chinese people aged 18 years and over

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A Global, Multi-center, Randomized, Double-Blind, Placebo-controlled, Phase III Clinical Study to Evaluate the Protective Efficacy, Safety and Immunogenicity of SARS-CoV-2 mRNA Vaccine in Population Aged 18 Years and Older

Rehabilitation for discharged COVID-19 patients: a randomized controlled clinical study

Randomized, Open, Multicenter Study on the Efficacy and Safety of Arbidol Hydrochloride Tablets in Treating Pneumonia in Patients Infected With Novel Coronavirus (2019-ncov).

An IIT, Randomized, Single-Center, Open-Lable, Standard Therapy Controlled Study to Evaluate the Efficacy and Safety of Colchicine Tablets in Patients With COVID-19

Medium-and long-term effects of Liu Zi Jue combined with Acupressure Therapy on pulmonary function and quality of life of discharged patients with severe COVID-19

Immunogenicity and Safety of the Heterologous Prime-boost Immunization With an Aerosolised Adenovirus Type-5 Vector-based COVID-19 Vaccine (Ad5-nCoV) After Two-dose Priming With an Inactivated SARS-CoV-2 Vaccine in Adults at 18 Years of Age or Above: a Randomised, Open-label, Parallel-controlled Clinical Trial

Rehabilitation assessment and treatment for Discharged COVID-19 patients

Safety and Immunogenicity of Sequential Immunization of Inactivated COVID-19 Vaccine and Recombinant COVID-19 Vaccine (Ad5 Vector) inChinese Healthy Adults Aged 60 and Above: a Randomized, Observer-blind,Parallel-controlled Clinical Trial

An Open-label,Phase IV Clinical Trial to Evaluate the Immunogenicity and Safety of the Inactivated SARS-CoV-2 Vaccine (Vero Cell) in Healthy Population Aged From 18 to 59 Years.

A Randomized, Controlled, Multicenter Phase 4 Clinic Trial to Evaluate the Immunogenicity and Safety of Combined Immunization of Inactivated SARS-CoV-2 Vaccines (Vero Cell) (COVAX) (Produced in Wuhan) and 23-valent Pneumococcal Polysaccharide Vaccine (PPV23) or Quadrivalent Inactivated Influenza Vaccine (IIV4)

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Efficacy and Safety of a Recombinant Neutralizing Human Anti-SARS-CoV-2 Monoclonal Antibody JS016 in Chinese Hospitalized Patients With SARS-CoV-2 Infection (COVID-19): a Multicenter, Randomized, Open-label, Controlled Trial

A Randomized, Double-Blinded Clinical Trial to Evaluate Additional Dose of Medium-dosage or High-dosage SARS-CoV-2 Inactivated Vaccine in Healthy Individuals Previously Vaccinated With CoronaVac

Randomized, Double-blind, Parallel-controlled Evaluation of the Immunogenicity Consistency and Safety of the Three Batches of Recombinant Novel Coronavirus Vaccine (CHO Cells) After Process Verification in the 18-59-year-old Population, and the Comparison With the Pilot-scale Batch Immunogenicity Non-inferiority Bridging and Safety Clinical

Follow-up investigation of COVID-19 lung function and rehabilitation training in discharged patients

An open-label, randomized, controlled clinical trial of Leflunomide in the treatment of COVID-19

Randomized, double-blind, placebo-controlled phase II clinical trial of immunogenicity, immunopersistence, and safety of graded Novel Coronavirus inactivated vaccine (Vero cells) in healthy persons aged 18 years and older

Open label study of the COVID-19 vaccine activating humoral immunity to promote nucleic acid negative conversion

A randomized, open-label, controlled clinical trial for azvudine in the treatment of new coronavirus delta

Clinical Study on the Combined Application of Hymecromone Tablets in the Treatment of SARS-CoV-2 Infection

Clinical Study on Immunogenicity and Safety of Recombinant Novel Coronavirus(COVID-19)Vaccine (CHO Cell) Combined With Tetravalent Influenza Virus Lysis Vaccine in People Aged 18 Years and Over

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A Phase IIIb Clinical Trial to Evaluate the Immunogenicity and Safety of One Heterologous Booster Dose of SARS-CoV-2 mRNA Vaccine in Population Aged 18-59 Years and 60 Years and Above Who Have Completed the 2-dose Series with Inactivated COVID-19 Vaccines

A Randomized and Controlled Phase IV Clinical Trial to Evaluate the Immunogenicity and Safety of COVID-19 Vaccine (Vero Cell), Inactivated Co-administrated With EV71 Vaccine (Vero Cell) in Children Aged 3-5 Years Old

A randomized, double-blind and placebo-controlled phase I/II clinical trial to explore the safety and immunogenicity of the COVID-19 DNA vaccine used as a boost dose to subjects previously immunized with COVID-19 inactivated vaccines

To evaluate the clinical mechanism of Lianhua Qingwen Capsule on the systemic immune regulation of patients with new coronavirus pneumonia

A Double-blind, Randomized Clinical Trial to Evaluate the Lot-to-lot Consistency, Immunogenicity and Safety of an Inactivated SARS-CoV-2 Vaccine (CoronaVac)Between Different Workshops for Prevention of COVID-19 in Healthy Children Aged 3-17 Years

Safety and Immunogenicity of Three Doses of an Inactivated SARS-CoV-2 Vaccine in Chinese Pulmonary Tuberculosis Patients Aged 18-75 Years: a Randomized, Double-blind, Parallel-controlled Clinical Trial

A Randomized, Open, Control Phase IV Clinical Trial, to Evaluate One Booster Immunization of Inactivated COVID-19 Vaccine (CoronaVac) or Original Vaccine in Adults Aged 18-45 Years Old Previously Vaccinated With Inactivated COVID-19 Vaccine From Difference Manufactures

A Randomized, Double-Blinded Clinical Trial to Evaluate the Immunogenicity Using Additional Dose of Medium-dosage or High-dosage COVID-19 Vaccine (Vero Cell), Inactivated in Populations Who Have Completed Primary Immunization 5-9 Months

A Safety, Tolerability, and Efficacy Study of IBI314 in Mild to Moderate Patients With COVID-19

A Phase I Clinical Trial to Evaluate the Safety, Tolerability and Preliminary Immunogenicity of the Candidate Recombinant SARS-CoV-2 Vaccine ChAdTS-S in Participants Aged 18 to 59 Years and Above

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A Randomized, Double-Blind, Placebo-Controlled Phase 1 Clinical Trial to Evaluate the Tolerability, Safety, Pharmacokinetic Profiles, and Immunogenicity of JS026 and JS026 in Together With JS016 Administered Intravenously to Healthy Chinese Subjects

Efficacy and Safety of Oral Lactobacillus Plantarum GUANKE (CGMCC NO.21720) in Enhancement of Antibody Level After SARS-CoV-2 Vaccination: A Prospective, Randomized, Double-blind, Placebo-Controlled Trial (Trial 1)

Efficacy and Safety of Oral Lactobacillus Plantarum GUANKE (CGMCC NO.21720) in Enhancement of Antibody Level After SARS-CoV-2 Vaccination: A Prospective, Randomized, Double-blind, Placebo-Controlled Trial (Trial 2)

A randomized, double-blind, placebo-controlled, multicenter clinical study of Jinshuibao in improving the symptoms related to pulmonary and cardiac function of COVID-19 during convalescence

Efficacy, safety and immunogenicity of a recombinant protein subunit vaccine (CHO cells) against COVID-19 in adults: an international multicenter, randomized, double-blind, placebo-controlled phase III study

A prospective, randomized, open, comparative, multi-center controlled trial to evaluate the safety, efficacy and immunogenicity of the third dose of vaccine reactivation in subjects that had received two doses of inactivated COVID-19 vaccine

SARS-CoV-2 mRNA Vaccine Heterologous Booster Immunization Strategy for Exploratory Clinical Trials (Yunnan)

Mindfulness Effects in COVID-19 Patients During the Coronavirus Disease 2019 (COVID-19) Pandemic: A Randomized Controlled Trial

A randomized, double-blind, placebo-controlled trial of a Phase II clinical study evaluating different doses of COVID-19 DNA vaccine in healthy adult and elderly subjects

Heterologous Prime-boost Immunization With an Aerosolised Adenovirus Type-5 Vector-based COVID-19 Vaccine (Ad5-nCoV) After Two-dose Priming With an Inactivated SARS-CoV-2 Vaccine in Adults Aged 18 Years and Above: a Multicenter, Open-label, Partially Parallel-controlled Clinical Trial

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A Randomized, Double-Blind, Placebo-Controlled, Bridging Study to Evaluate the Safety, Tolerability, and Pharmacokinetics of FB2001 in Healthy Subjects

Reactogenicity and Immunogenicity of Third Dose Vaccine Booster Following Two Doses of Inactivated Vaccines

A Randomized, Double-blind, Placebo-controlled Phase I/II Clinical Study to Evaluate the Safety, Tolerability and Immunogenicity of SCTV01C in Healthy Population Aged ≥18 Years Previously Unvaccinated Against COVID-19

A Phase II Double-blind, Placebo-controlled Study to Evaluate the Safety and Tolerability of AZD7442 in Chinese Adults

A study on the compliance and effectiveness of nursing exercise intervention on prone positioning in patients with mild and common COVID-19

The Respiratory Training for COVID-19 Patients: A Randomized Controlled Trial Study

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Primary Purpose	Suspension Flag
Treatment	0
Treatment	0
Treatment	0
Treatment	0
Treatment	0
Treatment	0
Treatment	0
Treatment	1
Treatment	0

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Treatment	1
Treatment	1
Treatment	0
Treatment	0
Treatment	0
Prevention	0
Treatment	0
Treatment	0
Prognosis/Quality of life	0
Prognosis/Quality of life	0

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Treatment	0
Prognosis/Quality of life	0
Prognosis/Quality of life	0
Treatment	0
Treatment	0
Treatment	0
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Treatment	0
Treatment	0
Treatment	0

For peer review only

Treatment	0
Treatment	0
Treatment	0
Treatment	0
Treatment	0
Treatment	1
Treatment	0
Treatment	1
Treatment	1
Treatment	0

For peer review only

Treatment	0
Treatment	0
Treatment	0
Prevention	0
Treatment	0
Treatment	0
Treatment	0
Treatment	0
Treatment	0
Treatment	0

For peer review only

Treatment	0
Treatment	0
Treatment	0
Treatment	0
Treatment	0
Treatment	0
Treatment	0
Treatment	0
Treatment	1
Treatment	1

For peer review only

Treatment	1
Treatment	1
Treatment	0
Treatment	0
Treatment	0
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Treatment	0
Treatment	0
Treatment	0

For peer review only

Treatment	0
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Treatment	0
Treatment	0
Prevention	1
Treatment	0
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For peer review only

Treatment	0
Treatment	1
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For peer review only

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Prevention	0
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For peer review only

Treatment	0
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For peer review only

Treatment	0
Treatment	0
Treatment	0
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Treatment	0
Treatment	1
Treatment	0
Prognosis/Quality of life	1
Treatment	0
Treatment	0

For peer review only

Treatment	1
Prevention	0
Treatment	0
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For peer review only

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For peer review only

Treatment	0
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Treatment	0
Treatment	1
Prognosis/Quality of life	0
Treatment	0
Treatment	0
Treatment	1

For peer review only

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Treatment	0
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Treatment	0
Prevention	0

For peer review only

Treatment	1
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Prevention	0

For peer review only

Treatment	1
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For peer review only

Treatment	0
Treatment	0
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Treatment	0
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Prognosis/Quality of life	0
Treatment	0

For peer review only

Treatment	0	
Treatment		0
Prevention	0	
Treatment	0	
Treatment	0	
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For peer review only

Prognosis/Quality of life	0
Prevention	0
Treatment	0
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For peer review only

Treatment	0
Treatment	0
Prognosis/Quality of life	0
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Treatment	0
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Treatment	0
Health Services Research	0
Treatment	0
Prevention	0
Treatment	0
Prevention	0
Prognosis/Quality of life	0
Treatment	0

For peer review only

Prevention	0
Prevention	0
Prevention	0
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For peer review only

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For peer review only

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Treatment	0
Treatment	0
Treatment	0
Prevention	0
Prognosis/Quality of life	0

For peer review only

Prognosis/Quality of life	0
Treatment	0
Treatment	0
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Prevention	0
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Prevention	0
Prevention	0

For peer review only

Prevention	0	
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Prevention		0
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Treatment	0	
Prevention		0

For peer review only

Prognosis/Quality of life	0
Prevention	0
Prevention	0
Prevention	0
Treatment	0
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Prevention	0

For peer review only

Treatment	0	
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For peer review only

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Prevention	0	
Treatment	0	

For peer review only

Treatment	0	
Treatment	0	
Treatment	0	
Prevention		1
Treatment	0	
Prevention	0	
Treatment	0	
Treatment	0	
Prevention	0	
Prevention	0	

For peer review only

Prevention	0
Treatment	0
Prevention	0
Prevention	0
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Prevention	0
Prevention	0

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Prevention	0	
Prevention		1
Prevention	0	
Treatment	0	
Treatment	0	
Prevention	0	
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Prevention	0	
Prevention	0	
Treatment	0	

For peer review only

Prevention	0
Treatment	0
Prevention	0
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Prevention	0
Prevention	0

For peer review only

Prevention	0
Prevention	0
Prognosis/Quality of life	0
Treatment	0
Treatment	0
Prevention	0
Prevention	0
Prevention	0
Prevention	0
Prevention	0

For peer review only

Prevention	0
Prognosis/Quality of life	0
Treatment	0
Treatment	0
Prognosis/Quality of life	0
Prevention	0
Prognosis/Quality of life	0
Prevention	0
Prevention	0
Prevention	0

For peer review only

Treatment	0
Prevention	0
Prevention	0
Prognosis/Quality of life	0
Treatment	0
Prevention	0
Treatment	0
Treatment	0
Treatment	0
Prevention	0

For peer review only

Prevention	0
Prevention	0
Prevention	0
Treatment	0
Prevention	0
Prevention	0
Prevention	0
Prevention	0
Treatment	0
Prevention	0

For peer review only

Treatment	0
Prevention	0
Prevention	0
Treatment	0
Prevention	0
Prevention	0
Prevention	0
Treatment	0
Prevention	0
Prevention	0

For peer review only

Treatment	0
Prevention	0
Prevention	0
Treatment	0
Treatment	0
Prognosis/Quality of life	0

For peer review only

Reason for Suspension (If any)	Recruitment status at last submission
#NOTREQ	Recruiting
#NOTREQ	Completed
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
Follow new disciplines on disease control from government	Withdrawn
#NOTREQ	Completed

For peer review only

The epidemic of COVID-19 has been controlled well at present, no eligible patients can be recruitted.	Suspended
The epidemic of COVID-19 has been controlled well in China, no eligible patients can be enrolled at present.	Terminated
#NOTREQ	Recruiting
#NOTREQ	Completed
#NOTREQ	Completed
#NOTREQ	Not yet Recruiting
#NOTREQ	Not yet Recruiting
#NOTREQ	Not yet Recruiting
#NOTREQ	Not yet Recruiting
#NOTREQ	Not yet Recruiting

For peer review only

#NOTREQ	Not yet Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting

For peer review only

#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
Canceled by the investigator	Not yet recruiting
#NOTREQ	Recruiting
the study was started in the second half of the outbreak, and the number of qualifying COVID-19 patients decreased with the control of the epidemic so that we had to stop our trial before reaching the predefined sample size	Terminated
Canceled by the investigator	Not yet recruiting
#NOTREQ	Recruiting

For peer review only

#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Completed
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting

For peer review only

#NOTREQ	Suspending
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
Retracted due to lack of patient	Suspending
Cancelled due to lack of patient	Suspending

For peer review only

Cancelled due to lack of patient	Recruiting
Cancelled due to lack of patient	Suspending
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting

For peer review only

#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
Canceled by the investigators	Suspending
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Suspending
Cancelled by the investigator	Not yet recruiting
#NOTREQ	Not yet recruiting

For peer review only

#NOTREQ	Active, not recruiting
Cancelled by the investigator	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Completed
#NOTREQ	Not yet recruiting
#NOTREQ	Completed

For peer review only

#NOTREQ	Completed
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting

For peer review only

#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting

For peer review only

#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
There were no eligible patients in the paticipated centers.	Suspended
#NOTREQ	Recruiting
Cancelled due to lack of patient	Suspending
#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting

For peer review only

Cancelled by the investigator	Suspending
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
Without CDE Approval	Withdrawn
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
Cancelled by the investigator	Suspending
#NOTREQ	Recruiting

For peer review only

#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
Cancelled by the investigator	Not yet recruiting
Cancelled by the investigator	Not yet recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting

For peer review only

#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Completed
Patients were transferred to designated hospitals for treatment as needed, the clinical trials cannot be conducted.	Withdrawn
#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
Cancelled by the investigator	Not yet recruiting

For peer review only

#NOTREQ	Not yet recruiting
#NOTREQ	Completed
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting

For peer review only

Cancelled by the investigator	Not yet recruiting
Cancelled by the investigator	Not yet recruiting
#NOTREQ	Not yet recruiting
Cancelled by investigator	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
Cancelled by the investigator	Suspending
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting

For peer review only

Cancelled by the investigator	Not yet recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
Cancelled by the investigator	Not yet recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
Cancelled by the investigator	Not yet recruiting

For peer review only

#NOTREQ	Not yet recruiting
#NOTREQ	Enrolling by invitation
Cancelled by the investigator	Not yet recruiting
#NOTREQ	Recruiting
Canceled by the investigator	Completed
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting

For peer review only

#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Suspending
#NOTREQ	Recruiting

For peer review only

#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
There were no eligible patients with CoVID-19 in the paticipated centers.	Suspended
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting

For peer review only

#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting

For peer review only

#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Active, not recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting

For peer review only

#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Active, not recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting

For peer review only

#NOTREQ	Active, not recruiting
#NOTREQ	Recruiting
#NOTREQ	Completed
#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Completed
#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting

For peer review only

#NOTREQ	Enrolling by invitation
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting

For peer review only

#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Active, not recruiting
#NOTREQ	Completed
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Completed
#NOTREQ	Recruiting
#NOTREQ	Completed

For peer review only

#NOTREQ	Not yet recruiting
#NOTREQ	Completed
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Active, not recruiting
#NOTREQ	No longer recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Active, not recruiting

For peer review only

#NOTREQ	Enrolling by invitation
#NOTREQ	Completed
#NOTREQ	Completed
#NOTREQ	Completed
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Active, not recruiting
#NOTREQ	Completed
#NOTREQ	Completed
#NOTREQ	Active, not recruiting

For peer review only

#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Completed
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting

For peer review only

#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Completed
#NOTREQ	Recruiting
#NOTREQ	Completed
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Active, not recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting

For peer review only

#NOTREQ	Completed
#NOTREQ	Not yet recruiting
#NOTREQ	Active, not recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Completed
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting

For peer review only

#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
Sponsor decided to cancel this trial.	Withdrawn
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Completed
#NOTREQ	Recruiting
#NOTREQ	Completed

For peer review only

#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Active, not recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting

For peer review only

#NOTREQ	Active, not recruiting
Canceled by the investigator	Suspending
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Completed
#NOTREQ	Recruiting
#NOTREQ	Active, not recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting

For peer review only

#NOTREQ	Not yet recruiting
#NOTREQ	Active, not recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Active, not recruiting

For peer review only

#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting

For peer review only

#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Completed
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Active, not recruiting

For peer review only

#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting

For peer review only

#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Active, not recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Active, not recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting

For peer review only

#NOTREQ	Active, not recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Completed
#NOTREQ	Recruiting
#NOTREQ	Completed
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting
#NOTREQ	Not yet recruiting
#NOTREQ	Recruiting

For peer review only

#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Recruiting
#NOTREQ	Completed

For peer review only

Study Start Date (Date of First Enrollment)	Estimated Total Enrollment	Number of Arms
2020/1/10	160	2
2020/1/26	80	2
2020/1/22	150	2
2020/1/29	48	2
2020/1/25	108	3
2020/1/30	30	2
2020/2/3	42	2
2020/2/5	40	2
2020/1/28	125	3

For peer review only

2020/2/12	308	2
2020/2/6	452	2
2020/1/29	45	3
2020/2/1	240	2
2020/2/1	284	2
2020/2/1	160	8
2020/2/4	100	2
2020/2/2	120	2
2020/2/3	100	2
2020/2/2	100	2

For peer review only

2020/2/3	100	2
2020/2/3	100	2
2020/2/3	90	3
2020/2/1	100	2
2020/2/1	400	4
2020/2/1	100	2
2020/1/29	90	3
2020/2/4	100	2
2020/2/4	140	4
2020/2/4	328	2

For peer review only

2020/2/10	100	3
2020/2/4	30	3
2020/2/4	30	3
2020/2/3	400	2
2020/1/31	300	3
2020/2/5	30	2
2020/2/1	60	3
2020/2/14	140	2
2020/2/5	30	2
2020/2/6	480	4

For peer review only

2020/1/31	70	2
2020/2/6	120	2
2020/2/1	400	2
2020/2/1	600	2
2020/2/6	160	2
2020/2/6	400	4
2020/2/6	30	2
2020/1/25	63	5
2020/2/10	100	2
2020/2/14	100	2

For peer review only

2020/2/10	60	2
2020/2/11	78	2
2020/2/12	112	2
2020/2/10	90	5
2020/2/11	200	2
2020/2/15	120	2
2020/2/15	238	2
2020/2/14	200	2
2020/2/15	60	3
2020/2/12	240	2

For peer review only

2020/2/13	240	4
2020/2/12	60	2
2020/2/13	408	2
2020/2/10	188	2
2020/2/12	60	2
2020/2/15	40	2
2020/2/11	40	2
2020/2/5	160	2
2020/2/14	160	2
2020/2/14	160	2

For peer review only

2020/3/1	60	2
2020/2/15	100	2
2020/2/17	120	2
2020/2/15	30	2
2020/2/20	320	4
2020/2/12	120	3
2020/2/14	72	2
2020/2/7	110	2
2020/2/17	45	2
2020/4/20	48	2

For peer review only

2020/2/16	60	2
2020/2/17	120	2
2020/2/1	60	2
2020/2/19	68	2
2020/2/16	20	2
2020/2/15	180	3
2020/2/15	520	2
2020/2/11	360	2
2020/2/10	300	2
2020/2/14	400	2

For peer review only

2020/4/23	142	2
2020/2/17	100	2
2020/2/17	100	2
2020/2/10	100	2
2020/3/1	200	2
2020/3/1	200	2
2020/4/30	300	3
2020/2/18	120	2
2020/2/17	40	2
2020/2/9	300	2

For peer review only

2020/2/21	186	2
2020/2/13	80	2
2020/2/20	300	2
2020/1/30	120	2
2020/2/18	72	3
2020/2/17	100	3
2020/2/20	40	2
2020/2/19	200	2
2020/2/15	240	2
2020/2/15	60	2

For peer review only

2020/2/19	480	3
2020/2/19	60	2
2020/2/3	200	2
2020/2/19	100	2
2020/2/4	60	2
2021/10/1	300	3
2020/2/20	60	3
2020/2/19	300	2
2020/2/22	100	2
2020/2/24	40	2

For peer review only

2020/2/20	120	2
2020/2/24	828	3
2020/2/7	132	2
2020/2/21	300	2
2020/2/1	24	2
2020/2/22	100	3
2020/2/10	460	2
2020/3/10	200	2
2020/2/23	40	2
2020/2/24	180	2

For peer review only

2020/3/1	40	2
2020/2/28	60	2
2020/2/24	180	6
2020/2/1	16	2
2020/2/15	348	2
2020/2/24	120	2
2020/2/25	39	2
2020/2/24	60	2
2020/2/25	20	2
2020/3/2	80	2

For peer review only

2020/2/17	60	2
2020/2/24	100	2
2020/2/25	60	2
2020/2/15	120	2
2020/3/5	45	2
2020/2/24	48	2
2020/2/24	60	2
2020/2/26	120	6
2020/1/22	80	3
2020/2/26	32	4

For peer review only

2020/2/22	186	2	
2020/2/20	240	2	
2020/3/1	300	2	
2020/2/22	60	2	
2020/2/27	20	2	
2020/3/1	26	2	
2020/2/24	30	3	
2020/2/27	204	2	
2020/3/4	292		2
2020/3/1	300	2	

For peer review only

2020/3/2	60	2
2020/3/5	90	3
2020/7/1	280	6
2020/2/29	40	2
2020/2/18	60	2
2020/2/27	340	2
2020/3/1	30	2
2020/3/1	80	2
2020/3/1	60	2
2020/3/2	80	2

For peer review only

2020/3/2	70	2
2020/3/1	60	2
2020/2/27	96	2
2020/3/2	394	2
2020/3/3	200	2
2020/2/26	120	2
2020/2/26	100	2
2020/3/3	328	2
2020/3/1	200	3
2020/3/1	16	2

For peer review only

2020/2/2	90	3
2020/3/5	60	2
2020/3/13	40	2
2020/2/29	60	2
2020/3/9	100	2
2020/2/20	100	2
2020/3/8	152	2
2020/2/4	300	3
2020/3/7	40	2
2020/2/1	60	2

For peer review only

2020/2/1	30	2	
2020/3/8		150	3
2020/3/16	240	4	
2020/3/17	140	2	
2020/3/9	60	2	
2020/3/10	60	2	
2020/2/15	50	2	
2020/3/10	40	2	
2020/2/10	50	2	
2020/2/12	80	2	

For peer review only

2020/3/1	120	2
2020/3/1	120	4
2020/2/15	70	2
2020/3/16	100	2
2020/2/1	128	2
2021/11/1	116	2
2020/3/6	40	2
2020/2/10	20	2
2020/2/20	100	4
2020/2/26	30	2

For peer review only

2020/2/15	400	4
2020/3/17	60	2
2020/3/31	108	3
2020/3/19	144	2
2020/9/1	20	2
2020/3/5	150	3
2020/3/23	204	2
2020/3/18	60	2
2020/3/23	60	2
2020/2/26	40	2

For peer review only

2020/3/23	1000	2	
2020/4/1	20	2	
2020/1/30	300	2	
2020/2/15	100		2
2020/2/21	90	5	
2020/3/1	#NaN	2	
2020/4/1	80	2	
2020/3/13	60	2	
2020/5/6		20	2
2020/4/1	80	2	

For peer review only

2020/1/25	56	2	
2020/4/1	210	2	
2020/4/1	750	2	
2020/4/1	100	2	
2020/4/1	150	2	
2020/4/12		500	3
2020/3/1	20	2	
2020/4/11	1456	44	
2020/4/20	120	2	
2020/2/11	300	2	

For peer review only

2020/4/16	744	12
2020/4/16	2000	2
2020/4/1	3808	4
2020/4/17	45	3
2020/4/22	50	4
2020/5/10	60	2
2020/4/25	160	2
2020/4/15	200	2
2020/5/1	200	2
2020/4/24	350	2

For peer review only

2020/2/10	128	2
2020/5/1	60	2
2020/3/1	242	2
2020/2/19	186	2
2020/4/20	200	2
2020/5/1	60	2
2020/4/29	2128	68
2020/3/26	2000	2
2020/5/2	120	2
2020/5/6	60	2

For peer review only

2020/5/8	60	2
2020/5/12	342	2
2020/5/22	422	4
2020/2/8	#NaN	2
2020/5/7	240	2
2020/5/8	200	2
2020/2/25	160	2
2020/2/12	60	2
2020/5/15	942	8
2020/2/15	120	2

For peer review only

2020/6/1	150	2
2020/2/15	60	2
2020/2/18	104	2
2020/1/21	300	2
2020/6/5	40	2
2020/3/2	30	2
2020/6/22	50	3
2020/2/10	93	2
2020/6/25	168	3
2020/7/12	900	6

For peer review only

2020/7/10	471	4	
2020/7/12	12	3	
2020/7/13	12	3	
2020/7/24	33	2	
2020/5/1	60	2	
2020/11/13	32	4	
2020/8/19	50	3	
2020/7/28	144	6	
2020/8/7	32	2	
2020/8/28	168	12	

For peer review only

2020/8/29	1600	2
2020/9/1	60	2
2020/9/14	45	2
2020/10/31	552	3
2020/10/1	120	2
2020/10/1	120	2
2020/9/24	481	9
2020/10/30	90	2
2020/8/1	200	2
2020/10/7	180	9

For peer review only

2020/10/31	198	2		
2020/10/28	120	5		
2020/2/28	111	2		
	2020/10/27	1000		12
2020/10/31	1040	3		
2020/11/17	720	4		
2020/12/29	150	3		
2020/12/4		960		2
2020/10/30	216	12		
	2020/11/17		960	16

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2020/12/2	640	4	
2020/11/30	29000	2	
2020/2/6	#NaN		6
2021/1/1	20	2	
2021/1/5	24	2	
2020/9/29	360	3	
2021/1/2	360	3	
2021/1/5	360	3	
2021/1/11	420	4	
2021/1/1	20	2	

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2021/1/1	120	2
2021/1/1	120	2
2021/1/14	40	4
2021/2/1	0	4
2021/1/29	200	2
2021/2/4	2200	2
2020/7/1	200	2
2021/6/20	48	3
2020/12/28	16	2
2021/3/23	480	2

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2021/3/23	480	8
2021/4/1	80	2
2021/4/7	120	4
2021/3/28	880	4
2021/2/22	180	6
2021/4/9	472	2
2021/4/20	840	22
2021/4/25	3580	15
2021/4/29	4400	18
2021/3/26	240	8

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2021/5/3	500	2	
2021/5/12	1152	3	
2021/5/14	1404	3	
2021/1/29	300	2	
3/1/2020	24	2	
6/3/2021	2016	9	
5/25/2021	300	4	
7/3/2021	400	2	
6/21/2021	20	2	
6/18/2021	100	2	

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8/1/2021	800	2
11/10/2020	42	6
7/9/2021	270	2
7/3/2021	75	6
7/12/2021	144	8
7/31/2021	1056	8
9/1/2021	1320	3
9/1/2021	480	2
8/1/2021	84	2
5/11/2021	1080	3

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8/15/2021	600	8
8/16/2021	150	6
8/1/2021	180	3
8/7/2021	600	2
8/7/2021	100	2
8/5/2021	2560	2
8/4/2021	360	2
7/28/2021	320	3
8/2/2021	1314	2
7/28/2021	144	6

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	7/22/2021	2000	2
	7/1/2021	102	3
2/8/2020		236	2
9/6/2021		60	2
2021/4/1		200	2
9/13/2021		420	3
2021/9/30		120	3
8/26/2021		300	4
6/19/2021		1400	3
5/6/2021		1404	3

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1/20/2021	200	2
11/1/2021	1200	2
9/9/2021	1680	4
2021/10/15	40	2
2021/10/1	60	2
2021/10/8	600	6
2021/11/3	60	2
2021/11/1	60	2
2020/7/25	300	2
9/3/2021	300	2

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2021/11/25	2000	4
11/5/2021	520	2
2021/12/25	480	6
2021/4/7	20	2
7/27/2021	2520	7
12/22/2021	240	3
12/20/2021	480	5
10/20/2021	340	2
12/31/2021	222	2
2021/5/24	60	2

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12/17/2021	48	7
3/1/2022	30	2
3/1/2022	40	4
2020/4/27	200	2
2021/9/2	14600	3
2021/8/2	200	3
2021/11/24	300	2
2021/10/16	66	2
2021/12/15	200	4
1/22/2022	10420	3

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12/29/2021	32	4
8/1/2021	400	4
12/1/2021	752	12
12/3/2021	272	2
2022/1/12	46	2
2020/1/1	89	2

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Arms
intervention group:Lopinavir-ritonavir tablets (each containing 200 mg of lopinavir and 50 mg of ritonavir), twice a day, 2 tablets at a time;Control group:Conventional standardized treatment;
Placebo Comparator: standard care standard care Intervention: Other: Standard care
Experimental: standard care + methylprednisolone therapy Methylprednisolone 40 mg q12h for 5 days
Experimental: Conventional medicines and TCMs granules Conventional medicines: oxygen therapy, antiviral therapy (alfa interferon via aerosol inhalation, and lopinavir/ritonavir, 400mg/100mg, p.o, bid) for 14 days. Traditional Chinese Medicines (TCMs) granules: one bag, p.o, bid, for 14 days
Experimental group:Methylprednisolone, intravenous injection, 1-2mg/kgÂ·d for 3 days.;Control group:Without any glucocorticoid therapy;
arm A:Ribavirin + Interferon alpha-1b;arm B:lopinavir / ritonavir + interferon alpha-1b;arm C:Ribavirin + LPV/r+Interferon alpha-1b;
Experimental: Darunavir, Cobicistat and conventional treatments After randomization, subjects take darunavir and cobicistat one tablet per day for 5 days, also take conventional treatments. Intervention: Drug: Darunavir and Cobicistat No Intervention: Conventional treatments
Experimental Group:Combined Treatment of Chinese medicine and western medicine;Control Group:Western medicine treatment;
Experimental: Observational group 5u washed microbiota suspension administered via nasogastric tube, nasojejunal tube or oral, combining with standard therapy. Intervention: Other: washed microbiota transplantation Placebo Comparator: Control group
Experimental: Group A-Standard treatment+lopinavir/ritonavir In group A, 50 cases are given ordinary treatment plus a regimen of lopinavir (200mg) and ritonavir (50mg) (oral, q12h, every time 2 tablets of each, taking for 7-14 days). Intervention: Drug: Lopinavir and Ritonavir Tablets Active Comparator: Group B-Standard treatment+arbidol

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Experimental: Remdesivir group
active remdesivir
Intervention: Drug: Remdesivir
Placebo Comparator: Control group
Placebos matched remdesivir
Experimental: Remdesivir group
active remdesivir
Intervention: Drug: Remdesivir
Placebo Comparator: Control group
Placebos matched remdesivir

A:Critical Treatment in Critical Period + Ankylosaurus;B:Critical Treatment in Critical Period + Ankylosaurus+M1 suppression therapy;C:Critical Treatment in Critical Period;

Routine treatment + Lianhua Qingwen Capsules/granules:4 capsules / time or 1 bag / time, 3 times a day;Conventional treatment:Treat as prescribed in the fourth edition of the diagnosis and treatment plan;

Routine treatment + Lianhua Qingwen Capsules/granules:4 capsules / time, 3 times a day;Conventional treatment:Treat as prescribed in the fourth edition of the diagnosis and treatment plan, without using traditional Chinese medicine;

Community exposure TCM intervention group:TCM intervention;Community exposure control group:Placebo;Hospital exposure TCM intervention group:TCM intervention;Hospital exposure control group:Placebo;Community suspected TCM intervention group:TCM intervention;Community suspected control group:Placebo;Hospital suspected TCM intervention group:TCM intervention;Hospital suspected control

control group:western medical therapies;Experimental group:Conventional medicine + TCM;

control group:basic western medical therapies;experimental group:TCM standard decoctions+basic western medical therapies;

experimental group:Pulmonary rehabilitation+Conventional treatment;control group:Conventional treatment;

experimental group:shadowboxing +conventional treatment;Control group:conventional treatments;

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experimental group:TCM decoctions+basic conventional therapy;Control group:basic conventional therapies;

experimental group :TCM decoctions+basic western medical therapies ;control group :basic western medical therapies ;

experimental group:Traditional Chinese Medicine+psychological intervention; Control group:Traditional Chinese Medicine;Control group:psychological intervention;

Experimental: Abidol hydrochloride
Standard symptomatic support therapy (SMT) plus abidol hydrochloride(0.2g, 3 times a day).
Intervention: Drug: Abidol hydrochloride
Experimental: Abidol Hydrochloride combined with Interferon atomization
No intervention: Symptomatic supportive treatment
Symptomatic supportive treatment
Experimental: Abidol hydrochloride was added on the basis of group I.
Abidol hydrochloride 0.2g once,3 times a day,2 weeks
Intervention: Drug: Abidol hydrochloride
Experimental: Methylprednisolone(<40mg/d)
Intervention: Drug: Methylprednisolone
Experimental: Methylprednisolone(40~80mg/d)
Intervention: Drug: Methylprednisolone
Intervention A:On the basis of routine medical treatment, the patients were given Novaferon Atomization inhalation, 20ug each time, twice a day, 7-14 days.;intervention B:On the basis of routine medical treatment, the patients were given lopinavir / ritonavir tablets (Kaletra) twice a day, twice each time, 7-14 days.;intervention C:On the basis of routine medical treatment, combined with Novafron and Kaletra, the usage and course of

treament group:Chinese medicine decoction;placebo group:placebo;

(ordinary) Western medicine group:Use the (ordinary) western medicine treatment plan in "the diagnosis and treatment plan of new coronavirus infection pneumonia" issued by the National Health Commission of the fourth edition.;(ordinary) Chinese and Western Medicine Group:Use the (ordinary) TCM clinical prescription combined with the western medicine treatment plan in "the diagnosis and treatment plan of new coronavirus infection

experimental group:conventional standardized treatment and Lopinavir-Ritonavir;control group:Conventional standardized treatment;

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DRV/c group:DRV/c (800mg/150mg QD) + Conventional treatment containing thymosin (1.6 mg SC QOD) ;LPV/r group:LPV/r (400mg/100mg bid) + Conventional treatment containing thymosin (1.6 mg SC QOD);other group:Conventional treatment containing thymosin (1.6 mg SC QOD) ;

Experimental group 1:current antiviral treatment+Baloxavir Marboxil tablets;Experimental group 2:current antiviral treatment+fabiravir tablets;Control group:current antiviral treatment;

A:BaloxavirMarboxil:80mg on day1,80mg on day4;and 80mg on day7 as neccessary. No more than 3 times administration in total.BaloxavirMarboxil:80mg on day1,80mg on day4;and 80mg on day7 as neccessary. No more than 3 times administration in total.;B:Favipiravir: 600 mg tid with 1600mg first loading dosage for no more than 14 days ;C:Lopinavir-Ritonavir: 2# (200mg/50 mg), twice daily, for 14days ;

Experimental group:Western medicine routine treatment combined with traditional Chinese medicine treatment;Control group:Western medicine routine treatment;

experimental group 1:Hydroxychloroquine 0.1 oral 2/ day;experimental group 2:Hydroxychloroquine 0.2 oral 2/ day;Placebo control group:Starch pill oral 2/ day;

control group :Conventional treatment ;Experimental group :conventional treatment combined with umbilical cord mesenchymal stem cell conditioned medium group;

Experimental: ASC09F+Oseltamivir
Intervention: Drug: ASC09F+Oseltamivir
Experimental: Ritonavir+Oseltamivir
Intervention: Drug: Ritonavir+Oseltamivir
Experimental: Oseltamivir
Experimental: VC
12g Vitamin C+sterile water for injection; total volume: 50ml. 12ml/h; infusion pump ; q12h.
Intervention: Drug: VC
Placebo Comparator: Sterile water for injection
50ml water for injection, 12ml/h; infusion pump; q12h

control group:Conventional treatment;experimental group:conventional treatment combined with umbilical cord blood mononuclear cells group;

Mild Type Group A:antiviral therapy and Chinese medicine treatment;Mild Type Group B:antiviral therapy, Chinese medicine treatment, and Novaferon atomization;Common and Heavy Type Group C:antiviral therapy and Chinese medicine treatment;Common and Heavy Type Group C:antiviral therapy, Chinese medicine treatment, and Novaferon atomization;

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Experimental group:Ruxolitinib combined with mesenchymal stem cell;control group: Routine treatment;

control group:basic western medical therapies;experimental group:Reduning injection combined with basic western medical therapies;

Control group:Health education+Basic treatment of western medicine;Experimental group:Health education+Basic treatment of Western medicine+Dialectical?treatment of traditional Chinese medicine;

Experimental group:health education, follow-up condition management by team of family doctors, Chinese medicine treatment;Control group:health education, follow-up condition management by team of family doctors;

Experimental group:Conventional standardized treatment and ASC09/Ritonavir;Control group:Conventional standardized treatment and Lopinavir/Ritonavir;

Low dose group:Shuanghuanglian 2 bottles/time, 3 times a day; Routine treatment.;Medium dose of group:Shuanghuanglian 4 bottles/time, 3 times a day; routine treatment.;High dose of group:Shuanghuanglian 6 bottles/time, 3 times a day; routine treatment.;Control group:Routine treatment;

Experimental: Hydroxychloroquine and conventional treatments
After randomization, subjects take hydroxychloroquine 400mg per day for 5 days, also take conventional treatments.
Intervention: Drug: Hydroxychloroquine
No Intervention: Conventional treatments

experimental group A:Conventional treatment followed by Intravenous infusion of Human Menstrual Blood-derived Stem Cells preparations;control gorup A:Conventional treatment;Experimental Group B1:Artificial liver therapy+conventional treatment;Experimental Group B2:Artificial liver therapy followed by Intravenous infusion of Human Menstrual Blood-derived Stem Cells preparations+conventional treatment;Control

Treatment group:baseline antiviral treatment (lopinavir / ritonavir or abidol) and inhaled with rSIFN-co;Control group:baseline antiviral treatment (lopinavir / ritonavir or abidol) and inhaled with interferon-a;

control group:Standard treatment;experimental group:Standard treatment and methylprednisolone for injection;

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experimental group:high-fow therapy by nasal cannulae (HFNC);control group:bag- valve mask oxygenation (SMO);

hydroxychloroquine group:oral intake hydroxycholoroquine 0.2 twice a day;control group:conventional therapy;

experimental group:Chloroquine Phosphate;control group:Lopinavir / Ritonavir;

General patients treated with normal treatment:Normal Treatment;General patients experimental group:Normal Treatment plus Sodium Aescinate for Injection;Severe patients control group 1:Normal treatment plus hormonotherapy;Severe patients control group 2:Normal Treatment;Severe patients experimental group:Normal Treatment plus Sodium Aescinate for Injection;

Experimental group:Traditional Chinese Medicine;control group:Basic treatment of modern medicine;

experimental group:routine treatment, Jinyebaidu granule, po, 1-2 bags once, 3 times a day;;control group:routinel treatment, treatment according to the fifth version of the diagnosis and treatment protocol, no use of traditional Chinese medicine.;

experimental group:Xiyanping injection;control group:alpha-interferon;

Experimental group:Conventional treatment and convalescent plasma therapy;Control group:Conventional treatment;

control group 1:Lopinavir / Ritonavir (Kaletra) and IFN aerosol inhalation;control group 2:Abidol and IFN aerosol inhalation.;Experimental group:ASC09/ Ritonavir (ASC09F) and IFN aerosol inhalation;

Experimental group:Hydroxychloroquin;Control group:Lopinavir /Ritonavir;

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Low-dose group:Low-dose hydroxychloroquine and conventional therapy;Medium-dose group:Medium-dose hydroxychloroquine and conventional therapy;High-dose group:High-dose hydroxychloroquine and conventional therapy;control group:conventional therapy;

Experimental group:Conventional treatent and hydroxychloroquine;Control group:Conventional treatment;

experimental group:TCM and general treatment;control group:general treatment;

control group:conventional therapy;Experience group:conventional therapy+tocilizumab;

experimental group:Diammonium Glycyrrhizinate Enteric-coated Capsules (oral, 150mg, Tid), Vitamin C tablets (oral, 0.5g, QD) and clinical standard antiviral treatment;control group:clinical standard antiviral treatment;

Experimental group:Routine treatment + Babaodan 6 capsules, bid orally;Control group:Conventional treatment;

experimental group:Polyinosinic-Polycytidylic Acid Injection and conventional therapy;control group:conventional therapy;

experimental group:Truncation and Torsion Formula and Routine treatment of Western Medicine;control group:Routine treatment of Western Medicine;

experimental group:Routine treatment and Shenqi Fuzheng Injection;control group:Routine treatment ;

experimental group:routine treatment and Kangbingdu granules;control group:routine treatment ;

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Control group :Western medicine routine treatment plan;Intervention group :Western medicine routine treatment plan plus TCM syndrome differentiation treatment;

Experimental group:TCM based on symptomatic treatment;Control group:Placebo TCM based on symptomatic treatment;

Control group:Western medicine basic treatment;Experimental group:Western medicine basic treatment combined with traditional Chinese medicine;

Experimental: NK Cells Treatment Group
Conventional treatment plus NK cells. Participants will receive conventional treatment plus twice a week of NK cells (0.1-2*10E7 NK cells/kg body weight).
Intervention: Biological: NK Cells
No Intervention: Conventional Control Group

A1:Hydroxychloroquine, small dose;A2:Hydroxychloroquine, high dose;B1:Abidol hydrochloride, small dose;B2:Abidol hydrochloride, high dose;

Thymosin treatment group:Thymosin for injection 1.6 mg sc qd for 5 days;PD-1 treatment group:Camrelizumab 200 mg single dose, diluted to 100 ml intravenous infusion;Conventional treatment group:Conventional treatment;

Experimental group:Conventional Treatment & Tanreqing Capsules (oral, 3 capsules at a time, 3 times a day);Control Group:Conventional Treatment;

Experimental group:honeysuckle decoction;Control group:placebo;

Phosphoric chloroquine:2 tablets phosphoric chloroquine BID, complicated with therapy recommended by "New Coronavirus Infected Pneumonia Diagnosis and Treatment Scheme (revision of Trial Version 5)";Placebo:2 tablets placebo BID;

Experimental: UC-MSCs Treatment Group
Conventional treatment plus UC-MSCs:

Participants will receive conventional treatment plus 4 times of UC-MSCs(0.5*10E6 UC-MSCs/kg body weight intravenously at Day 1 , Day 3 , Day 5 , Day7)

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Experimental: Group A
Treatment group
Interventions:
Drug: Bromhexine Hydrochloride Tablets
Drug: Arbidol Hydrochloride Granules

Phosphoric chloroquine:2 tablets phosphoric chloroquine BID, complicated with therapy recommended by New Coronavirus Infected Pneumonia Diagnosis and Treatment Scheme (revision of Trial Version 5);Placebo:2 tablets placebo BID, complicated with therapy recommended by New Coronavirus Infected Pneumonia Diagnosis and Treatment Scheme (revision of Trial Version 5);

Experimental group:Regulating intestinal flora + conventional treatment;Control group:conventional treatment;

control group:conventional therapy and placebo;Experience group:conventional therapy and ALA;

Experimental group:Oral administration of 5 tablets of 1mg Azvudine tablets daily;Control group:According to the "Pneumonia Diagnosis and Treatment Plan for New Coronavirus Infection (Trial Version 5)" issued by National Health Commission of People's Republic of China, the subjects were given corresponding treatment.;

Chinese traditional medicine group 1:Traditional Chinese medicine qingfei prescription treatment;Chinese traditional medicine group 2:Traditional Chinese medicine clear lung prescription and compound houttuynia mixture treatment;Control group:Conventional treatment;

Experimental group:Carrimycin;Control group:Lopinavir and Ritonavir Tablets;

Experimental group:oral hydroxychloroquine sulfate tablets;Control group:Conventional treatment meet the Guideline;

experimental group:Truncated Torsion' Formula and Routine treatment of Western Medicine;control group:Routine treatment of Western Medicine;

Experimental: MP group
Intervention: Drug: Methylprednisolone
No Intervention: Con group

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Experimental: FZHY Group
usual treatment (respiratory function rehabilitation training + Vitamin C tablets)

Fuzheng Huayu tablets
Interventions:

hydroxychloroquine sulfate:Day1: first dose: 6 tablets(0.1g/table), second dose: 6 tablets (0.1g/table)after 6h ;Day2~5: 2 tablets (0.1g/table), BID;phosphate chloroquine:Day 1-3:500mg BID; Day 4-5:250mg BID;

hydroxychloroquine sulfate:Day1: first dose: 6 tablets(0.1g/table), second dose: 6 tablets (0.1g/table)after 6h ;Day2~5: 2 tablets (0.1g/table), BID;phosphate chloroquine:Day1-3:500mg,BID Day4-5:250mg,BID;

Conventional treatment group:The conventional treatment group will be treated according t o the guidance of the â€œDiagnosis and Treatment Scheme of COVID-19â€ published by the National Health Commission.;Experimental group:conventional treatment combined wit h Chloroquine Phosphate.;

experimental group :Traditional Chinese medicine compound granules + western medicine symptomatic treatment ;control group:western medicine symptomatic treatment ;

experimental group :Traditional Chinese medicine compound granules + western medicine symptomatic treatment;control group :western medicine symptomatic treatment ;

Control group:Treated according to the New Protocol for Diagnosis and Treatment of NCP(5th edition), without using traditional Chinese medicine;Experimental group 1:Basic treatment+Low dose of honeysuckle oral liquid 60ml each time, tid;Experimental group 2:Basic treatment + high dose of honeysuckle oral liquid 180 ml each time, tid;

Experimental group:Health Guidance + Guixi Regulating Lung Gong Method + Shenling Baizhu Powder + Moxibustion Treatment;Control group:Health Guidance + Guixi Regulating Lung Gong Method ;

Experimental group:ultra short wave diathermy, 10 minutes twice daily for 12 consecutive days;control:standard medical therapy;

Experimental group:atic treatment medicines;Control group:Symptomatic treatment medicines;

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Control group:Conventional treatment;Experimental group:Conventional treatment pius Dao Yin;

Experimental group:Chloroquine Phosphate;Control group:No;

experimental group.:Recombinant Human Interferon a1b Eye Drops ;Control group:placebo drop;

experimental group:mesenchymal stem cells;control group:saline;

Kesuting syrup group:conventional therapy and Kesuting syrup, p.o., 20ml, three times a day;Keqing capsule group:conventional therapy and Keqing capsule, p.o., three pills once, three times a day;control group:conventional therapy;

Chloroquine phosphate gloup:Chloroquine phosphate1.0gx2 days for the first dose, 0.5gx12 day from the third day ; Hydroxychloroquine sulfate gloup:Hydroxychloroquine sulfate 0.2g bid x 14 days;Routine treatment group:Recommended treatment plan for novel coronavirus pneumonia severe and critical cases;

group A:Arbidol Hydrochloride Tablets, Liushen Capsule, standard therapy;control group:standard therapy;

Control group:Conventional treatment;Experimental group:Conventional treatment +acupressure+Liuzijue qigong;

Experimental group:Basic treatment combined Triazavirin;Control group:Basic treatment combined Triazavirin placebo;

control group:conventional therapy;Experience group:conventional therapy and Tranilast;

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Group 1:Traditional Chinese Medicine prescription No. 1 and 2;Group 2:Gankeshuangqing capsule;Group 3:Shuanghuanglian oral liquid;

Experimental group:ozonated autohemotherapy;Control group:Standard medical treatment;

Experimental group:rhG-CSF, 5ug/kg, combinated with standard treatment;Control group:standard treatment;

Experimental group:Anti-SARS-CoV-2 virus inactivated plasma;Control group:Ordinary plasma;

1:inhalate the mycobacterium vaccae for injection;2:inhalate Physiological saline;

Experimental: Yin Hu Qing Wen Decoction Group
Based on the standard western medicine treatment, the patients will be given Yinhu Qingwen Decoction (Granula) for 10 days.
Interventions:
Drug: YinHu QingWen Decoction
Low-dose group:Tablets; 200mg; oral; twice a day; The adult dose is 1600 mg per time on first day; the duration of treatment will be 10 d.;Middle-dose group:Tablets; 200mg; orally; twice a day;The adult dose is 1800 mg per time on first day; the duration of treatment will be 10 d.;High-dose group:Tablets; 200mg; oral; twice a day; The adult dose is 2400 mg per time on first day; the duration of treatment will be 10 d.;

Experimental group:pulmonary training;control group:Conventional medication;

Experimental group:Conventional therapy and Pediatric Huatanzhike granules;Control group:Conventional therapy;

Experimental group:PD-1 mAb + Standard treatment;Control group:Standard treatment;

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Phosphoric chloroquine:2 tablets phosphoric chloroquine BID, complicated with therapy recommended by New Coronavirus Infected Pneumonia Diagnosis and Treatment Scheme (revision of Trial Version 5);Placebo:2 tablets placebo BID, complicated with therapy recommended by New Coronavirus Infected Pneumonia Diagnosis and Treatment Scheme (revision of Trial Version 5);

Intervention group 1:Oral antiviral oral solution (Xiangxue Pharmaceutical);Intervention group 2:Oral "Wu-Zhi-Fang-Guan-Fang" decoction;Control group:Routine quarantine measures;

Experimental group:TCM+Routine treatment of Western Medicine;Control group:Routine treatment of Western Medicine;

Experimental:Conventional treatment and Shenfu injection;control:Conventional treatment;

Experimental: rhACE2 group
0.4 mg/kg IV BID for 7 days (unblinded) + standard of care
Intervention: Drug: Recombinant human angiotensin-converting enzyme 2 (rhACE2)
No Intervention: Control group
Standard of care: no placebo

Hydroxychloroquine sulfate gloup:Hydroxychloroquine sulfate 0.2g bid x 14 days a day ;Chloroquine phosphate gloup:The first dose of chloroquine phosphate was 1gx2 days, and the third day was 0.5gx12 days ;Control group:Recommended treatment plan for novel coronavirus pneumonia diagnosis and treatment plan ;

Routine treatment group:Conventional treatment of 2019-nCoV pneumonia;Dipyridamole treatment group:Conventional treatment of 2019-nCoV pneumonia and Dipyridamole;

Experimental group :Oral leflunomide;Control group:Oral placebo;

experimental group:dihydroartemisinin piperaquine tablets combined with antiviral treatment.;control group:antiviral treatment was selected as: a-interferon and Ardibdol;

Psychological Intervention Group:"Intelligent Psychosomatic Regulation System" Intervention;Control Group :General Treatment ;

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Experimental group:Iv injection of Wharton's Jelly mesenchymal stem cells (1Ã—10^6/kg), cell suspension volume: 40ml;Control group:Iv 40ml saline;

Experimental group:Conventional treatent and adalimumab;Control group:conventional treatment ;

control group with common NCP:conventional treatment;experimental group with common NCP:conventional treatment combined with ozone therapy;experimental group with severe NCP:conventional treatment combined with ozone therapy;control group with severe NCP:conventional treatment;experimental group with critical NCP:conventional treatment combined with ozone therapy;control group with critical NCP:conventional treatment;

Two groups:Different stem cell doses;

Combination therapy group:Xiyanping injection+ conventional treatment;Conventional treatment group:Lopinavir / Ritonavir tablets, alpha-interferon;

Experimental group:Traditional Chinese medicine soup;Control group:Western antiviral;

Experimental group:High-dose Vitamin C;Control group:Routine treatment;

Experimental group:Intravenous injection of human umbilical cord mesenchymal stem cells (UC-MSC);Control group:Routine treatment + placebo;

Control group:Routine treatment (Pure western medicine): Lopinavir-ritonavir tablet combined with recombinant human interferon alpha 2b injection;Investigation group:(Integrated Chinese and western medicine): Lopinavir-ritonavir tablet combined with recombinant human interferon alpha 2b injection(routine treatment)+Qing-Wen Bai-Du-Yin formula granules;

Experimental Group:Conventional Treatment & Low Dose Recombinant Human Interleukin-2, i.m.(1 million IU per time, once another day, 3 times per week, 2 weeks);Control Group:Conventional Treatment+Placebo i.m.(once another day, 3 times per week, 2 weeks);

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Experimental group:umbilical cord mesenchymal stem cells;control group:convention treatment;

Experimental group:Routine treatment + plasma treatment;Control group:Routine treatment;

Experimental group:Lopinavir and Ritonavir Tablets;control group:Routine symptomatic support treatment;

Experimental group:Syndrome differential treatment of Chinese Medicine + Western medicine basic treatment;Control group:Western medicine basic treatment;

Experimental: Human Umbilical Cord-Mesenchymal Stem Cells (UC-MSCs)
Participants will receive standard of care plus 3 does of UC-MSCs
Intervention: Biological: UC-MSCs
Placebo Comparator: Placebo
Participants will receive standard of care plus 3 does of placebo
Experimental: UC-MSCs treatment group
Participants will receive conventional treatment plus 4 times of 0.5*10E6 UC-MSCs /kg body weight intravenously at Day1, Day3, Day5, Day7).
Intervention: Biological: UC-MSCs
Placebo Comparator: Control group

experimental group:Health education+ pulmonary rehabilitation;control group:Health educatio;

Experimental group 1:Routine treatment + Kanguan No. 1;Control group (Mild patients):Routine treatment;Experimental group 2:Routine treatment + Kanguan No. 2;Control group (general patient):Routine treatment;Experimental group 3:Routine treatment + Kanguan No. 3;Control group (severe patients):Routine treatment;

Experimental group of ordinary COVID-19:Lopinavir / ritonavir tablets combined with Xiyanping injection;Control group of ordinary COVID-19:Keep ritonavir/ritonavir treatment;Experimental group of severe COVID-19:Lopinavir / ritonavir tablets combined with Xiyanping injection;

1 the critical group :injecting Mesenchymal stem cells;2 the severe group:injecting Mesenchymal stem cells;3 the control group of the critical group:injecting normal saline;4 the control group of the severe group:injecting normal saline;

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Experimental group:Conventional treatment & Acupuncture;Control Group:Conventional treatment;

Experimental group:Farpiravir tablets;Control group:Abidole tablets;

Routine treatment + Jing-Yin Granule:Jing-Yin Granule 2 bags / time, 3 times a day ;Routine treatment :Treat as prescribed in the sixth edition of the diagnosis and treatment plan, without using traditional Chinese medicine;

Experimental group:Danoprevir sodium tablets,/ritonavir oral;Control group:Symptomatic treatment;

Experimental group:enteral nutrition emulsion, tpf-t 200ml Bid;Control group:No intervention;

Exocrine group:Aerosol inhalation of exosomes;Control group:Blank;

High inflammation oXiris CRRT treatment group:oXiris CRRT treatment;High inflammation Conventional treatment group:Conventional treatment;Low inflammation conventional treatment group:Conventional treatment;

experimental group:TCM formula and general treatment based on "Diagnosis and Treatment Program for COVID-19" (trial version 6th);control group:general treatment based on "Diagnosis and Treatment Program for COVID-19" (trial version 6th);

Pirfenidone group:Conventional standard treatment + Pirfenidone 400mg three times a day;Control group:Conventional standard treatment;

The experimental group:Wear a self-made "gastroscope mask" during gastroscopy;Control group:Without self-made "gastroscope mask" during gastroscopy;

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Experimental group:Acetylcysteine inhaled via tracheal tube;Control group:Placebo (saline) inhaled via tracheal tube;

Control group:conventional therapy;CIK group:Umbilical cord blood CIK cells (1.6Ã—10E8/kg) were injected twice every other day.;NK group:Umbilical cord blood NK cells (1.6Ã—10E8/kg) were injected twice every other day.;

Experimental: DAS181 SD group Cohort 1, Stage 1
DAS181 SD group 4.5mg/day for 7 or 10 days
Intervention: Drug: DAS181
Experimental: DAS181 HD group Cohort 1, Stage 1
DAS181 HD group 9mg/day for 7 or 10 days

Experimental group:Conventional treatment and anti-SARS-CoV-2 virus inactivated plasma;Control group:Conventional treatment and Ordinary plasma;

Experimental group:On the basis of routine treatment, add 100ml of normal saline and 50ml of Xuebijing injection twice a day;;Control group:conventional therapy;

Experimental group:Capsules of bismuth potassium citrate, 2 capsules at a time, once every 12 hours; take warm water with breakfast and dinner; do not take high protein foods such as milk at the same time.;Control group:Simulated capsules of bismuth potassium citrate, 2 capsules at a time, once every 12 hours; take warm water with breakfast and dinner; do not take high protein foods such as milk at the same time.;

Experimental group:Combined standard therapy of chloroquine phosphate aerosol inhalation solution;control group:Water for injection atomization inhalation combined with standard therapy;

Group 1:Eight lung exercises for rehabilitation;Group 2:Free activities;

Control group:Conventional integrated traditional Chinese and western medicine treatment and western medicine psychological intervention treatment;Experimental group:Conventional integrated traditional Chinese and western medicine treatment, western medicine psychological intervention therapy and traditional chinese medicine psychological intervention therapy;

Case series:rehabilitation and Lung eight-segment exercise;Control group:Keep the initial life style;

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Group 2:SMO Assisted tracheal intubation;Group 1:Assisted tracheal intubation via nasal high-flow oxygen inhalation;

Control group:TCM + Conventional medicine treatment + conventional medicine's psychological intervention;Experimental group:Integrated Chinese and Conventional medicine treatment + conventional medicine's psychological intervention + TCM psychological intervention;

Control group:Coventional Treatment;Experimental group:Conventional treatment+LIUSHENWAN;

Lipoic acid group:Routine Therapy and administration of Lipoic acid injection;Blank control group:Routine Therapy and administration of placebo;

1:meditation+Standard treatment of traditional Chinese and Western Medicine;2:Standard treatment of traditional Chinese and Western Medicine;

Positive drug control group:Refer to the National Health Office Medical Letter (2020) 77 document "Diagnosis and treatment plan for the novel coronavirus disease (trial version 5)" general treatment plan. Based on the general treatment plan of Western medicine + Lianhua Qingwen Capsule (0.35 g per capsule) orally, 4 capsules each time;Compound Yuxingcao Mixture Group:Refer to the National Health Office Medical Letter (2020) 77 Control group:Refer to the National Health Office Medical Letter (2020) 77 document "Diagnosis and treatment plan for the novel coronavirus disease (trial version 5)" general treatment plan.;Experimental group:Refer to the National Health Office Medical Letter (2020) 77 document "Diagnosis and treatment plan for the novel coronavirus disease (trial version 5)" general treatment plan. Based on the general treatment plan of western

Interferon Therapy Group:Standard treatment + cerrokin (recombinant human interferon alpha 1beta) 10UG BID is administered by nebulization for 10 days.;Control group:Standard treatment + blank;

Early corticosteroid intervention group:Early corticosteroid intervention;Middle-late corticosteroid intervention group:Middle-late corticosteroid intervention;Control group:No corticosteroid;

treatment group :Ba duanjin;

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treatment group1:HUMSCs: intravenous infusion, 5 A— 10E7 cells / time, once / week, twice / course;treatment group 2:HUMSCs: intravenous infusion, 5 Ã— 107 cells / time, 1 time / week, 2 times / course, a total of 2 courses; Exosomes: intravenous administration, 180mg / time, 1 time / day, 7 days / course, 2 courses in total;control group:The control group was given the same amount of placebo (stem cell solvent);

Experimental: Tetrandrine Cohort
After the subjects were enrolled, they were given "Tetrandrine 60mg QD" for a course of 1 week(Take 6 days, stop using for 1 day)
Intervention: Drug: Tetrandrine
No Intervention: Control Cohort

experimental group:Intravenous infusion of NK cells on day 0,3 and 6 , 1*107â€”1*108/kg body weight of NK cells each day.;control group:Sodium Potassium Magnesium Calcium and Glucose Injection.;

Experimental group:Intravenous drip, 0.2 g (4 vials) once daily for adults; slow drip after dilution with 250 ml of 5% Glucose Injection or 0.9% Sodium Chloride Injection, not less than 120 minutes or 30-40 drops/minute. Continuous administration for 7 days, observation until 21 days or discharge;Control group:Conventional treatment, no control drug;

experimental group:Ma-Xing-Gan-Shi decoction and Conventional Western Medicine Treatment;control group:placebo and Conventional Western Medicine Treatment;

experimental group:ebastine 10mg bid, nterferon-alpha aerosol inhalation 5million U bid and Lopinavir 200 mg, 2 capsules at a time, bid;control group:interferon-alpha aerosol inhalation 5million U bid and Lopinavir 200 mg, 2 capsules at a time, bid;

CRRT group:CRRT;Control group:Routine treatment, CRRT only when there are emergency indications;

Control group:Basic treatment refers to the treatment according to the diagnosis and treatment plan of the health care Commission (Sixth Edition). (after three days of medication, if the symptoms cannot be controlled in time and effectively, the patients can be sent out of the group immediately to receive the treatment&#;Experimental group 1:Basic treatment + honeysuckle oral liquid at a low dose of 60ml / time, three times a

Experimental group:Lung rehabilitation training+Conventional treatment;Control group:Conventional treatment;

experimental group:Based on the standard treatment recommended in the guidelines, a combination of tozumab and adamumab was used;control group:Follow the guidelines for standard treatment;

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Experimental group:Convalescent plasma therapy + routine treatment;Control group:routine treatment;

Experimental: Favipiravir Combined With Tocilizumab group
Favipiravir: On the 1st day, 1600mg each time, twice a day; from the 2nd to the 7th day, 600mg each time, twice a day. Oral administration, the maximum number of days taken is not more than 7 days.

Group A:Sevoflurane anesthesia maintenance, common extubation;Group B:Sevoflurane anesthesia was maintained, and the extubation strategy of "circulatory induced respiration" was adopted after the operation.;Group C:Propofol anesthesia maintenance, common extubation;Group D:Propofol anesthesia was maintained, and the extubation strategy of "circulatory induced respiration" was adopted after the operation.;

Experimental: Bevacizumab Group
Intervention: Drug: Bevacizumab
No Intervention: Control Group

experimental group:Based on the standard treatment recommended in the guidelines, a combination of Prolongin (Enoxaparin Sodium Injection) was used.;control group:Follow the guidelines for standard treatment.;

experimental group:Based on the standard treatment recommended in the guidelines, a combination of Prolongin (Enoxaparin Sodium Injection) was used.;control group:Follow the guidelines for standard treatment.;

eperimental group:Conventional treatment and convalescent plasma therapy;control group:conventional treatment;

experimental group:Ixekizumab and antiviral therapy;control group:antiviral therapy;

Experimental group:Bufonis venenum injection;Control group:0.9% N.S.;

Experimental group:chloroquine phosphate ;Control group:none;

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Group 2:Western medicine conventional treatment;Group 1:TCM treatment;

common type :Immunotherapy with recombinant chimeric DC vaccine;Latent period group:Immunotherapy with recombinant chimeric DC vaccine;Normal control group:Immunotherapy with recombinant chimeric DC vaccine;Blank vaccine control group:0.9%NS;

control group:Routine supportive treatment;experimental group:TCM decoction intervention + routine supportive treatment;

Experimental group:Conventional treatent and Ulinastatin;Control group:Conventional treatent ;

Exocarpium Citri Grandis Group:Oral Administration of Exocarpium Citri Grandis Phlegm Cough Solution;Placebo Group:Oral Administration of Placebo;

Experimental: Yinhu Qingwen Granula Group
Based on the standard medical treatment, the patients will be given Yinhu Qingwen Granula for 10 days.
Interventions:
Drug: Yinhu Qingwen Granula

Experimental group:Pirfenidone;Control group:Blank;

Control group:Routine medicines and food;Experimental group:Taking Newgen beta-gluten probiotic composite powder;

Tricholoma matsutake group:Taking tricholoma matsutake ;Cannabis sativa capsule group:Taking cannabis sativa capsule ;Dendrobium candidum group:Taking dendrobium candidum;Routine treatment group:Taking routine drugs and food;

A:Long-acting interferon alpha-2a (135ug) + ribavirin;B:Abidol + ribavirin;

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Suspected case treatment group:TCM formula 1 or TCM formula 2;Suspected case control group:null;Confirmed case treatment group:Western medicine+(TCM formula 3 or TCM formula 4 or TCM formula 5 or TCM formula 6);Confirmed case control group:Western medicine;

Experimental group:Anti-SARS-CoV-2 virus inactivated plasma;Control group:Ordinary plasma;

Group 1:Liuzijue exercise;Group 2:Respiratory muscle training;Group 3:Liuzijue exercise combined with respiratory muscle training;

Experimental group:On the basis of rehabilitation treatment, the subjects in the experimental group take Gu-Shen Ding-Chuan-Wan;Control group:standard treatment;

Experimental group:On the basis of the current clinical treatment of SNCP, NK cells and MSCs were increased;Control group:current clinical treatment of SNCP;

Experimental: favipiravir tablets+chloroquine phosphatetablets tablets group
favipiravir tablets+chloroquine phosphatetablets tablets
Intervention: Drug: favipiravir tablets+chloroquine phosphatetablets tablets
Experimental: favipiravir tablets group
favipiravir tablets

Experimental group:Hua-Shi Bai-Du granules;control group:conventional treatment of western medicine;

Experimental group:normal treatment and auricular point pressing;control group:normal treatment;

Group 1:psychological support;Group 2:cognitive behavioral therapy;

Pirfenidone group:Support treatment with oral taken Pirfenidone capsule;Support treatment group:Support treatment ;

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Experimental group vs control group:Hydroxychloroquine versus Placebo;

Experimental: Bone Marrow-Derived Mesenchymal Stem Cells (BM-MSCs)
Conventional treatment plus BM-MSCs
Intervention: Biological: BM-MSCs
Placebo Comparator: Placebo
Conventional treatment plus placebo

Experimental treatment group:Oral chloroquine phosphate tablets (treatment group);control group:Oral placebo group (control group);

Experimental: oxyhydrogen
conventional treatment + hydrogen/ oxygen inhaled
Intervention: Device: oxyhydrogen
Experimental: oxygen
conventional treatment + oxygen inhaled

Experimental: NK cells
The NK cells are going to be give by intravenous infusion (10E8 cells per kilogram of body weight, once a week) .
Intervention: Biological: NK cells,IL15-NK cells,NKG2D CAR-NK cells,ACE2 CAR-NK cells,NKG2D-ACE2 CAR-NK cells
Active Comparator: Control group
conventional symptomatic treatments such as antiviral (oseltamivir), hormones, oxygen therapy, mechanical ventilation and other supportive therapies
Interventions:
Drug: Oseltamivir

Control group:regular treatment;Experimental group:Compound amino acid combined with routine treatment;

Group M:Play sleep-aid music on the cell phone at the same time as basic therapy, listen to it once every night at 22 o'clock, and the patient falls asleep with the music. A total of 2 weeks of treatment.;Group C:No music, the use of basic treatment, in accordance with the "new coronavirus infection pneumonia diagnosis and treatment program" trial fifth, sixth, seventh version of the provisions of the implementation ;

Experimental: hDPSCs group
Routine treatment + Intravenous injection of human dental pulp stem cells
Intervention: Biological: allogeneic human dental pulp stem cells (BSH BTC & Utooth BTC)
Placebo Comparator: Control group
Routine treatment + Intravenous saline injection (Placebo)

Nintedanib group:Nintedanib;Control group:Placebo;

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experimental group:Rabeprazole plus chloroquine;control group :Lopinavir + rabeprazole;

Experimental: Favipiravir group
On the 1st day, 1600mg each time, twice a day; from the 2nd to the 7th day, 600mg each time, twice a day. Oral administration, the maximum number of days taken is not more than 14 days.
Intervention: Drug: Favipiravir

Experimental: Anluohuaxian combined with regular treatment group
Anluohuaxian: 6g each time, twice a day
Intervention: Drug: Anluohuaxian
No Intervention: regular treatment group

Control:NA;Noninvasive cardiopulmonary monitoring group:Wear non-invasive multi-channel cardiopulmonary monitor;

Experimental Group:Xiaotan Sanjiefang granule;Control Group:placebo (contain 5% test drug);

Active Comparator: Arm 1
1×10^11vp of Ad5-nCoV administered through 1.0 mL intramuscular injection in the deltoid muscle on Day 0
Intervention: Biological: Recombinant novel coronavirus vaccine (Adenovirus type 5 vector)
Active Comparator: Arm 2

NK cell group:NK cells were injected by intravenous drip once a day for 2 to 3 times in total, each dose was 4 10^7 pieces /kg body weight,100ml normal saline suspension;Control group:100ml normal saline intravenous drip;

Phase I A1:Low dose;Phase I A2:Placebo;Phase I A3:Middle dose;Phase I A4:Placebo;Phase I A5:High dose;Phase I A6:Placebo;Phase I E1:Low dose;Phase I E2:Placebo;Phase I E3:Middle dose;Phase I E4:Placebo;Phase I E5:High dose;Phase I E6:Placebo;Phase I F1:Low dose;Phase I F2Placebo;Phase I F3:Middle dose;Phase I F4:Placebo;Phase I F5:High dose;Phase I F6:Placebo;phase II A1:Low dose;phase II

Experimental group:exercise rehabilitation under remote monitoring;Control group:patient education;

experimental group:"Guangdong Pneumonia NO.1"+basic treatment; control group:basic treatment and(or) antiviral therapy;

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Experimental: Emergency schedule & Two doses of medium dosage vaccine
24 participants in phase Ⅰand 60 participants in phase Ⅱ will receive two doses of medium dosage inactivated SARS-CoV-2 vaccine at the emergency vaccination schedule,and 60 participants in phase Ⅱ will receive one dose of booster immunization with medium dosage vaccine 6 months after the emergency schedule

experimental group:Chinese herbal tea;control group:drinking water;

control group:General basic epidemic prevention measures;sachet group:General basic epidemic prevention measures + Traditional Chinese medicine sachet;Oral and nasal spray group:General basic epidemic prevention measures + Oral and nasal spray;Oral Chinese medicine group:General basic epidemic prevention measures + Traditional Chinese medicine for epidemic prevention in high risk groups;

1.5ATA:1.5ATA HBO;2.5ATA:2.5ATA HBO;Control group:Routine treatment;

Experimental group: mild cases:Ulinastatin 300,000 IU, Q12H;Experimental group: severe or critical cases:Severe cases: 1 million IU of ulinastatin, Q8H; Critical cases: ulinastatin 1.6 million IU, Q8H;Control group: mild cases:conventional therapy;Control group: severe or critical cases :conventional therapy;

Experimental: Moxibustion plus Cupping
Moxibustion plus cupping and basic therapy (rehabilitation direction and basic breathing exercise) once a day for 4 weeks (28 sessions).The moxibustion plus cupping treatments were divided into 2 alternating formulas:

Control Group:Qimaifeiluoping Mixture Simulator + Respiratory Training + Rehabilitation Education Guidance;Experimental group:Qimaifeiluoping Mixture + Respiratory Training + Rehabilitation Education Guidance;

Experimental group:Shengmai Decoction;Control group:Shengmai Decoction simulant;

Experimental group:Shumian capsule;Control group:Shumian capsule placebo;

Experimental group:Basic treatment + kolimycin;Control group:Basic treatment;

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No intervention: the NCP standard treatment group
According to the diagnosis and treatment guidelines, the patients were divided into four types: mild, common, severe and critically ill. Then patients with common and severe ill were randomly divided into two groups, respectively, namely the NCP standard treatment group and the NCP aspirin group (aspirin 100 mg/d, oral + combined standard treatment)

Experimental group:Jiexingjunzi granules ;control group:placebo;

Group 2:Aliskiren with Standard Care;Group 1:Nifedipine with Standard Care;

Group 2:Conventional treatment;Group 2:Conventional treatment plus liu-zi--jue exercisee therapy;

Control group:xiaoyao capsule;Experimental group:xiaoyao capsule;

experimental group:Perform respiratory rehabilitation exercises.;control group:No respiratory rehabilitation exercise;

Phase I A1:Low dosage;Phase I A2:Placebo;Phase I A3:medium dosage;Phase I A4:Placebo;Phase I A5:high dosage;Phase I A6:Placebo;Phase I F1:Low dosage;Phase I F2:Placebo;Phase I F3:medium dosage;Phase I F4:Placebo;Phase I F5:high dosage;Phase I F6:Placebo;Phase I G1:low dosage;Phase I G2:placebo;Phase I G3:medium dosage;Phase I G4:placebo;Phase I G5:high dosage;Phase I

Experimental group:Administration of hydroxychloroquine sulfate;control group:Placebo given;

control group:conventional therapy;Experimental group:Bufei Huoxue Capsule, 1.4g/day, three times a day, three months of treatment;

Control group:the basic treatment withThymosin enteric capsule + Pien Tze Huang Capsules placebo;Experimental group:the basic treatment with Thymosin enteric capsule +Pien Tze Huang Capsules;

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Routine treatment group:Routine treatment;WMT group:Washed microbiota transplantation;

Experimental Group:FNC+Standard of Care;Control Group:FNC dummy tablet+ Standard of Care;

Experimental: Experimental Vaccine-medium dosage
24 participants at medium dosage stage in phase I will receive two doses of primary immunization according to the immunization schedule of day 0, 28 and will receive 1 dose of booster immunization 1 year after primary immunization; 100 participants at medium dosage stage in phase II will receive two doses of primary immunization according to the

Experimental: berberine group (B group)
Patients in the B group were given berberine hydrochloride tables 0.3g tid orally or tube feed daily, until the 14th day of the study. Other treatments include general support therapy, oxygen therapy, antiviral drugs, in combination with antibiotics and small doses of glucocorticoids if necessary, nutritional and organ function support

Experimental group:Taking artemisinin-piperaquine;Control group:Symptomatic treatment with non-antiviral drugs;

Experimental group:Lu-Dang-Shen oral solution 10ml / time, 2 times a day;Control group:Placebo 10ml / time, 2 times a day;

Group A:self-service pulmonary rehabilitation;Group B:Internet-assisted pulmonary rehabilitation;

Experimental group:standard treatment + shuanghuanglian treatment;Control group:standard treatment;

Experimental: Low dosage vaccine on a 0- and 28-day schedule
Two doses of low dosage Inactivated SARS-CoV-2 Vaccine at the vaccination schedule of day 0, 28
Intervention: Biological: Low dosage Inactivated SARS-CoV-2 Vaccine on a 0- and 28-day schedule

intervention group:abdominal massage + ST-36 (Zusanli) acupressure;control group:regular nursing;

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Experimental group:Western medicine(pirfenidone)+chinese herb;control:Pirfenidone drugs + traPirfenidone + placebo of traditional Chinese Medicine;

Experimental Group:Baicoli chewable tablets, 200mg / tablet, dosage 200mg / Time, 3 times a day, total 600mg / day, total course of treatment: 10 days;Control Group:Placebo 200 mg / day, 3 times a day, 600 mg / day, for 10 days;

Experimental group:Conventional treatment plus Liu--zi-jue exercise therapy;Control group:Conventional treatment;

Control:Guidelines for routine treatment;Therapy group:TCM;

Experimental: Test group
Intervention: Combination Product: JS016 (anti-SARS-CoV-2 monoclonal antibody)
Experimental: Control group
Intervention: Combination Product: JS016 (anti-SARS-CoV-2 monoclonal antibody)

Experimental group:Bromhexine Hydrochloride Tablets + Standard treatment;Control group:Standard treatment;

Experimental: Population I
Population I has 20 subjects and is considered negative for SARS-COV-2 and fluorescent RT-PCR nucleic acids. Population I is intramuscular injection of deltoid muscle of upper arm with low dose of vaccine.
Intervention: Biological: Recombinant new coronavirus vaccine (CHO cell) low-dose group

Ninety-three eligible participants selected by cluster sampling are randomized to an intervention group (n = 47) and a control group (n = 46) using a computerized random number generator by a trial statistician who had no clinical involvement in the project. Participants in the control group receive routine treatment according to the Chinese Management Guidelines for COVID-19, while participants in the intervention group receive

Low dose group:Investigational vaccine or placebo of corresponding dose;Medium dose group:Investigational vaccine or placebo of corresponding dose;High dose group:Investigational vaccine or placebo of corresponding dose;

Experimental: Population I
In population I, there were 150 subjects who injected with 2 doses of low-dose test vaccine into the deltoid muscle of the upper arm according to the 0 and 1 month immunization schedule.
Intervention: Biological: Recombinant new coronavirus vaccine (CHO cell) low-dose group

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Experimental: Low dosage vaccine on a 0- and 28-day schedule
Two doses of low dosage Inactivated SARS-CoV-2 Vaccine at the vaccination schedule of day 0, 28
Intervention: Biological: Low dosage Inactivated SARS-CoV-2 Vaccine on a 0- and 28-day schedule

Experimental: Conort 1
BRII-196 dose level 1 or placebo
Interventions:
Drug: BRII-196
Drug: Placebo

Experimental: Conort 1
BRII-198 dose level 1 or placebo
Interventions:
Drug: BRII-198
Drug: Placebo

Experimental: anti-SARS-CoV-2 monoclonal antibody(SCTA01)
SCTA01: single dose on Day0
Intervention: Biological: SCTA01
Placebo Comparator: Placebo
Placebo: single dose on Day0

Experimental group:Sancai granule;Control group:stimulant;

Group 1:Atomization treatment, 0.2mg IFN-k/ 0.5 mg TFF2;Group 2:Atomization treatment, 1mg IFN-k/ 2.5 mg TFF2;Group 3:Atomization treatment, 2mg IFN-?/ 5 mg TFF2;Group 4:Atomization treatment, 4mg IFN-?/ 10 mg TFF2;

Experimental: Population I
Population I has 20 subjects and is considered negative for SARS-COV-2 and fluorescent RT-PCR nucleic acids. Population I is intramuscular injection of deltoid muscle of upper arm with low dose of vaccine.
Intervention: Biological: Biological/Vaccine: Recombinant new coronavirus vaccine (CHO

Experimental: Low-dose, 18-55 years of age
Intervention: Biological: BNT162b1
Experimental: High-dose, 18-55 years of age
Intervention: Biological: BNT162b1
Placebo Comparator: Placebo, 18-55 years of age

Experimental: MW33
Intervention: Combination Product: MW33 injection
Experimental: Placebo
Intervention: Combination Product: MW33 injection placebo

Experimental: Middle-dose vaccine (18-55 years) & Two dose regimen
Two doses of middle-dose experimental vaccine at the schedule of day 0, 28.
Intervention: Biological: Two doses of middle-dose recombinant SARS-CoV-2 vaccine (Sf9 Cell) at the schedule of day 0, 28
Experimental: Middle-dose vaccine (≥ 55 years) & Two dose regimen

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Intervention group:Mindfulness-based interventions (MBIs) ;Control group:Health education;

Experimental group:vaccination;Placebo control group:vaccination;

experimental group:Vaccine INO-4800;control group:placebo;

Experimental: Experimental Vaccine-low dosage
low dosage inactivated SARS-CoV-2 vaccine
Intervention: Biological: Two doses of low dosage inactivated SARS-CoV-2 vaccine at the schedule of day 0,28
Experimental: Experimental Vaccine-medium dosage

Jianpi Kanggu Group:Jianpi Kanggu Granules;Jintiange Group:Jintiange Capsule;

Jianpi Kanggu Granules Group:Jianpi Kanggu Granules;Xianling Gubao Capsule Group:Xianling Gubao Capsule;

Experimental: MID A
20 participants(18-49), Ad5-nCoV , two doses, Intramuscular administration
Intervention: Biological: Recombinant Novel Coronavirus Vaccine (Adenovirus Type 5 Vector)
Placebo Comparator: MID B
Active Comparator: Anti-SARS-CoV-2 Monoclonal Antibody dose 1/2/3
use Anti-SARS-CoV-2 Monoclonal Antibody,dose 1/2/3 to treat COVID-19
Intervention: Biological: Recombinant Human Anti-SARS-CoV-2 Monoclonal Antibody(25mg/kg;50mg/kg;100mg/kg)
Placebo Comparator: Placebo

Jiexingjunzi granule group:Jiexingjunzi granules;Qibei Fuzheng granule formation:Qibei Fuzheng granule;

A1: 3 doses, medium dosage or placebo; A2: 3 doses, high dosage or placebo; A3: 2 doses, medium dosage/placebo; A4: 2 doses, high dosage/placebo; B1: medium dosage/placebo; B2: high dosage/placebo

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Experimental: experimental Group
the experimental arm will receive hydrogen-oxygen mixed gas inhalation (Hydrogen-Oxygen Generator with Nebulizer, AMS-H-03, output: 3 L/min (hydrogen concentration: 66.7%, oxygen concentration: 33.3%)) ,the treatment duration will be 8 hours per day, for 12 weeks

A:Vaccinated with 5ug vaccine or placebo;B:Vaccinated with 10ug vaccine or placebo;C:Vaccinated with 15ug vaccine or placebo;D:Vaccinated with 20ug vaccine or placebo;E:Vaccinated with 25ug vaccine or placebo;

experimental group:Personalized psychological intervention plus narrative exposure therapy;control group:Personalized psychological intervention;

A1:medium dosage;A2:high dosage;A3:placebo;A4:medium dosage;A5:high dosage;A6:placebo;A7:medium dosage;A8:high dosage;A9:placebo;B1:medium dosage;B2:high dosage;B3:placebo;

Experimental: Aged 26-45, Commercial Scale
Commercial scale inactivated SARS-CoV-2 vaccine in adults aged 26-45 years.
Intervention: Biological: Two doses of commercial scale inactivated SARS-CoV-2 vaccine at the schedule of day 0,14
Experimental: Aged 18-59, Pilot Scale

Experimental group:0,14 day immunization procedure;Control group:0,14 day placebo of immunization procedure;Experimental group:0, 21 day immunization procedure;Controll group (0,21 day immunization procedure):0,21 day placebo of immunization procedure;

Active Comparator: MW33 injection-1200mg
Intervention: Combination Product: MW33 injection
Active Comparator: MW33 injection-2400mg
Intervention: Combination Product: MW33 injection
Placebo Comparator: Placebo
Experimental: BNT162b2 18-85 years of age
Intervention: Biological: BNT162b2
Placebo Comparator: Placebo 18-85 years of age
Intervention: Other: Placebo
Experimental: Middle-dose vaccine (18-59 years) & Two dose regimen
Two doses of middle-dose experimental vaccine at the schedule of day 0, 14.
Intervention: Biological: Two doses of middle-dose recombinant SARS-CoV-2 vaccine (CHO Cell) at the schedule of day 0, 14
Experimental: Middle-dose vaccine (18-59 years) & Three dose regimen
Experimental: Low-dose vaccine (18-59 years) & Two dose regimen
two doses of low-dose Recombinant COVID-19 vaccine (Sf9 cells) at the schedule of day 0, 21.
Intervention: Biological: Low-dose Recombinant COVID-19 vaccine (Sf9 cells) (18-59 years) & Two dose regimen

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A1 Adult group:INO-4800 1.0 mg / placebo;A2 Elderly group:INO-4800 1.0 mg / placebo;B1 Adult group:INO-4800 2.0 mg/ placebo;B2 Elderly group:INO-4800 2.0 mg/ placebo;

Two cohorts:Test vaccine and placebo control;

Experimental: moderate COVID-19 group
patients with moderate COVID-19 receiving western medicine treatment
Intervention: Drug: Alpha-interferon alpha, abidol, ribavirin, Buzhong Yiqi plus and minus formula, Huhuang Detoxicity Paste, Baimu Qingre Jiedu Paste, fumigation/inhalation of vitamin C

Control Group:Standard care;Aerosol inhalation:Standard care plus aerosol inhalation;

Experimental: Cohort 1
BRII-196 and BRII-198 dose level 1 or placebo
Interventions:
Drug: BRII-196 and BRII-198
Drug: Placebo

0, 14 emergency vaccination group:0, 14 day immunization procedure;0, 21 emergency vaccination group:0, 21 day immunization procedure;0, 28 emergency vaccination group:0, 28 day immunization procedure;

0, 14 emergency vaccination group:two doses of vaccine at the schedule of day 0, 14;0, 21 emergency vaccination group:two doses of vaccine at the schedule of day 0, 21;0, 28 emergency vaccination group:two doses of vaccine at the schedule of day 0, 28;

0,14 emergency vaccination group:two doses of vaccine at the schedule of day 0,14;0, 21 emergency vaccination group:two doses of vaccine at the schedule of day 0, 21;0,28 emergency vaccination group:two doses of vaccine at the schedule of day 0, 28;

A:COVID-19 mRNA vaccine 5ug/0.5ml;B:COVID-19 mRNA vaccine 10ug/0.5ml;C:COVID-19 mRNA vaccine 15ug/0.5ml;D:Placebo;

Experimental group:Routine treatment + Lian-Hua Qing-Ke Tablets;Control group:Conventional treatment;

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Experimental group:Routine treatment + Lian-Hua Qing-Ke Tablets;Control group:Conventional treatment;

Experimental group:Lian-Hua Qing-Wen Capsule/Granule;Control group:Isolated observation;

JMB2002 5mg/Kg group:5mg/Kg dose of JMB2002 single intravenous infusion;JMB2002 15mg/Kg group:15mg/Kg dose of JMB2002 single intravenous infusion;JMB2002 15mg/Kg group:30mg/Kg dose of JMB2002 single intravenous infusion;JMB2002 50mg/Kg group:50mg/Kg dose of JMB2002 single intravenous infusion;

Experimental: adults group (aged 18-59 years) & vaccine
three doses of 40μg Recombinant COVID-19 vaccine (Sf9 cells) at the schedule of day 0, 21, 42.
Intervention: Biological: Recombinant COVID-19 vaccine (Sf9 cells)
Placebo Comparator: adults group (aged 18-59 years) & placebo

Experimental group: Qing-Jin Yi-Qi granule + rehabilitation treatments; Control group: rehabilitaion treatments;

Lian-Hua Qing-Wen Capsule Group:4 capsules/time, 3 times a day;Isolated observation control group:In accordance with the management requirements of the "New Coronavirus Pneumonia Prevention and Control Plan (Seventh Edition)" issued by the State Council, a 14-day quarantine medical observation was given;

Experimental group:Shugan Jieyu Capsules;Control group:Shugan Jieyu Capsule Simulator;

Experimental: BRII-196 and BRII-198 in adult subjects with severe COVID-19
Intervention: Drug: BRII-196 and BRII-198
Experimental: BRII-196 and BRII-198 in adult subjects with mild-moderate COVID-19
Intervention: Drug: BRII-196 and BRII-198
Experimental: Placebo in adult subjects with mild-moderate COVID-19

inactivated vaccine group:Novel Coronavirus inactivated vaccine (Vero cell);control group:Nil;

Experimental: Combined immunization group
The combined immunization group is randomly divided into two subgroups, 120 subjects in each group. The combined immunization subgroup I receive SARS-CoV-2 inactivated vaccine (Vero cell)&Quadrivalent Influenza Vaccine on day0 and SARS-CoV-2 inactivated vaccine (Vero cell) (second dose) on day 28 The combined immunization subgroup II

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Experimental: Middle-dose vaccine (18-59 years)
Three doses of middle-dose experimental vaccine at the schedule of day 0, 28, 56
Intervention: Biological: a middle-dose recombinant COVID-19 vaccine (CHO Cell) (18-59 years) at the schedule of day 0, 28, 56
Experimental: High-dose vaccine (18-59 years)

Trial group:Qingfei Paidu Granule;Control group: Conventional treatment;

Experimental: "0-28 days" vaccine group
One dose of recombinant Ad5 vectored COVID-19 vaccine on day 0, the second dose of subunit vaccine (ZF2001) against COVID-19 on day 28, and a third of subunit vaccine (ZF2001) against COVID-19 on month 4.
Interventions:
Group X: Two-dose 10-ug youth (18-59 years) group and two-dose 25-ug youth (18-59 years) group:All the subjects are administrated V-01 or placebo on Days 0 and Day 21 at the deltoid muscle in the upper arm and tested for safety, tolerance and immunogenicity as per the schedule described in the study protocol.;Group Y: Two-dose 10-ug elder (>=60 years) group and two-dose 25-ug elder (>=60 years) group:All the subjects are
Group A: 10-ug youth (18-59 years) group:All subjects are given one dose of V-01 or placebo on Days 0 and 21.;Group B: 25-ug youth (18-59 years) group:All subjects are given one dose of V-01 or placebo on Days 0 and 21.;Group C: 50-ug youth (18-59 years) group:All subjects are given one dose of V-01 or placebo on Days 0 and 21.;Group D: 10-ug elder (>=60 years) group:All subjects are given one dose of V-01 or placebo on Days 0

Intervention group:vaccinated in the morning;Control group:vaccinated in the afternoon;

Experimental: A1a Phase I low 2 doses
Ad5-nCoV containing 0.5E10 vp, 2 doses 56 days apart, Aerogen Solo
Intervention: Biological: Recombinant Novel Coronavirus Vaccine (Adenovirus Type 5 Vector) for Inhalation
Placebo Comparator: A1b Phase I placebo low 2 doses
Experimental: Phase 1 low-dose group
Intervention: Biological: low-dose Recombinant SARS-CoV-2 Vaccine (CHO cell)
Experimental: Phase 1 high-dose group
Intervention: Biological: high-dose Recombinant SARS-CoV-2 Vaccine (CHO cell)
Placebo Comparator: Phase 1 placebo group
Experimental: A1 aged ≥ 71
300 subjects age ≥ 71 （A1）receive 3 doses of vaccine
Intervention: Biological: 3-doses schedule 1 of COVID-19 Vaccine (Vero Cell), Inactivated
Experimental: A2 aged ≥ 71
200 subjects are ≥ 71 （A2）receive 3 doses of vaccine
Low dose adult group: mRNA COVID-19 vaccine/Placebo; Medium-dose adult group: mRNA COVID-19 vaccine/Placebo; Secondary high-dose adult group: mRNA COVID-19 vaccine/Placebo; High dose adult group: mRNA COVID-19 vaccine/Placebo; Low dose older group: mRNA COVID-19 vaccine/Placebo; Medium-dose older group: mRNA COVID-19 vaccine/Placebo; Secondary high-dose older group: mRNA COVID-19 vaccine/Placebo;

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Experimental: Experimental Group
Participant will receive two doses of medium dosage inactivated SARS-CoV-2 vaccine . Vaccine will given by intramuscular injection on day 0 and day 28.
Intervention: Biological: SARS-CoV-2 Inactivated Vaccine
Placebo Comparator: Control Group

Experimental group: 1st dose : combined vaccination of COVIV+IIV4; 2nd dose: combined vaccination of COVIV+PPV23; Control group A: 1st dose: COVIV only; 2nd dose: COVIV only; Control group B: 1st dose: IIV4 only; 2nd dose: PPV23 only;

Experimental group: 1st dose : combined vaccination of COVIV+PPV23 ; 2nd dose: combined vaccination of COVIV+IIV4; Control group 1: 1st dose: COVIV only; 2nd dose: COVIV only; Control group 2: 1st dose: PPV23 only; 2nd dose: IIV4 only;

treatment group:Qingjin Yiqi granule + comprehensive rehabilitation therapy of traditional Chinese and Western Medicine;Control group:comprehensive rehabilitation therapy of traditional Chinese and Western Medicine;

Experimental: nicotinamide plus usual care
Intervention: Drug: nicotinamide
No Intervention: usual care

Biological: Recombinant Novel Coronavirus Vaccine (Adenovirus Type 5 Vector) 0.5ml;Biological: Recombinant Novel Coronavirus Vaccine (Adenovirus Type 5 Vector) 0.3ml

Experimental: heterologous boost arm with Ad5 vectored vaccine
Subjects who have been primed with two doses of inactive SARS-CoV-2 vaccine will receive a booster of recombinant SARS-CoV-2 Ad5 vectored vaccine after 3~6 months.
Intervention: Biological: Recombinant SARS-CoV-2 Ad5 vectored vaccine
Active Comparator: homogeneous boost arm with inactive vaccine

Experimental group:DelNS1-2019-nCoV-RBD-OPT1;Control group:Placebo;

inactivated vaccine group:inactivated Novel Coronavirus Pneumonia vaccine (Vero cells);mRNA vaccine group:Novel Coronavirus Pneumonia mRNA vaccine (DF104B1);

Treatment Group of Xinguan Fuzheng Zhuanyin Decoction :Western medicine standard treatment combined with Xinguan Fuzheng Zhuanyin Decoction;Placebo group:Western medicine standard treatment;

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Experimental: SCB-2019 vaccine

Investigational SCB-2019 vaccine contains 30 μg of SCB-2019 antigen, and 1.5 mg CpG 1018 and 0.75 mg Alhydrogel as adjuvants, in each 0.5 mL dose

Intervention: Biological: Candidate Vaccine, SCB-2019

Placebo Comparator: Placebo

Experimental: 30mg LY-CovMab

LY-CovMab is diluted with 0.9% sodium chloride to a total volume of 250 mL for IV infusion

Intervention: Drug: LY-CovMab

Experimental: 150mg LY-CovMab

LY-CovMab is diluted with 0.9% sodium chloride to a total volume of 250 mL for IV infusion

Experimental: Experimental Group

180 subjects (including 90 adults aged 18-59 years and 90 elderly aged 60 year and older) will receive two doses of inactivated COVID-19 vaccine on day 0 and day 28

Intervention: Biological: Inactivated COVID-19 Vaccine

Placebo Comparator: Control Group

Experimental: Population I

Population I has 20 subjects aged 12 to 17 and is considered negative for SARS-COV-2 and fluorescent RT-PCR nucleic acids. Populationlis intramuscular injection of deltoid muscle of upper arm with vaccine.

Intervention: Biological: Recombinant new coronavirus vaccine (CHO cell) group

Experimental: Adult Group 1a

Adult healthy subjects (18 to 59 years of age, inclusive) receive 202-CoV low adjuvant dose at Day 0 and Day 28

Intervention: Biological: 202-CoV low adjuvant dose

Experimental: Adult Group 1b

Experimental: Adult Group 2a

Adult healthy subjects (18 to 59 years of age, inclusive) receive 202-CoV low adjuvant dose at Day 0 and Day 28

Intervention: Biological: 202-CoV low adjuvant dose

Experimental: Adult Group 2b

Biological: Experimental Group1

The COVID-19 vaccine was manufactured by Sinovac Research & Development Co., Ltd,and the quadrivalent influenza vaccine and the 23-valent pneumococcal polysaccharide vaccine(PPV23) were manufactured by Sinovac Biotech Co. The COVID-19 vaccine:600SU inactivated virus in 0.5 mL of aluminium hydroxide solution per

Experimental: candidate vaccine

Intervention: Biological: SARS-CoV-2 Vaccine (Vero Cells), Inactivated

Placebo Comparator: Placebo

Intervention: Biological: Placebo

Biological: SARS-CoV-2 Vaccine (Vero Cells), Inactivated

2 doses of SARS-CoV-2 Vaccine (Vero Cells), Inactivated should be administered as an intramuscular injection into the lateral deltoid of the upper arm with a 28-day interval.

Biological: Placebo

2 doses of Placebo should be administered as an intramuscular injection into the lateral

Experimental: Inactivated SARS-CoV-2 vaccine Lot 1

Participants (n=360) aged 26-45 years will receive Inactivated SARS-CoV-2 vaccine Lot 1 according to 0,28-day immunization schedule.

Intervention: Biological: Inactivated SARS-CoV-2 Vaccine (Vero cell)

Experimental: Inactivated SARS-CoV-2 vaccine Lot 2

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Experimental: Low-dose vaccine (6-11 years)
three doses of low-dose Recombinant COVID-19 vaccine (Sf9 cells) at the schedule of day 0, 21 , 42.
Intervention: Biological: Recombinant COVID-19 vaccine (Sf9 cells)
Experimental: Medium-dose vaccine (6-11 years)

Adults (18-59 years old) A:They were randomly assigned to trial group and placebo group in a ratio of 4:1;Adults (18-59 years old) B:They were randomly assigned to trial group and placebo group in a ratio of 4:1;Adults (18-59 years old) C:They were randomly assigned to trial group and placebo group in a ratio of 4:1;Elderly group (>=60 years old) A:They were randomly assigned to trial group and placebo group in a ratio of 4:1;Elderly group (>=60

1:Internal administration of rehabilitation traditional Chinese medicine, kung fu exercise;2:Internal administration of rehabilitation traditional Chinese medicine;3:No;

control group:Ganmai Dazao Decoction;Experimental group:Dialectical governance based on three parties;

Experimental group:Basic treatment + Fuzheng detoxification granules;Control group:Basic treatment;

Experimental group:Using Fuzheng Yiqing decoction;Parallel control group:Without using Fuzheng Yiqing decoction;External literature data control:None;

18-59 years:Investigational vaccine or placebo at the dose of 5×10^10vp;>=60 years:Investigational vaccine or placebo at the dose of 5×10^10vp;

Phase I (Phase I):Novel coronavirus mRNA vaccine 15ug/0.5ml;Phase II (Phase II) Group A:Novel coronavirus mRNA vaccine 15ug/0.5ml;Phase II (Phase II) Group B:Placebo (normal saline);

Arm A:prompt vaccination strategies;Arm B:deferred vaccination strategies;

Adult low-dose group:Investigational vaccine or placebo of corresponding dose;Elderly low-dose group:Investigational vaccine or placebo of corresponding dose;Adult medium-dose group:Investigational vaccine or placebo of corresponding dose;Elderly medium-dose group:Investigational vaccine or placebo of corresponding dose;Adult high-dose group:Investigational vaccine or placebo of corresponding dose;Elderly high-dose

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test group:mRNA vaccine 15ug/0.5ml;Control group:Placebo 0.5ml;

rehabilitation group:telerehabilitation plus health education ;TCM group:Traditional Chinese medicine plus health education ;control group:health education;

Experimental: Arbidol tablets + basic treatment
Interventions:
Drug: Arbidol
Other: basic treatment
Sham Comparator: basic treatment
Experimental: Colchicine group
The colchicine treatment includes an initial dose of 1 mg (1 mg and 0.5 mg two hours after), followed by 0.5 mg every 12 hours during the next 6 days and 0.5 mg every 24 hours until the completion of 10 days of total treatment.

Control group:conventional treatment;TCMR group:acupressure+Qigong exercise+conventional treatment;

Active Comparator: Inactivated vaccine group
Subjects who have been vaccinated with two doses of inactivated SARS-CoV-2 vaccine will receive one dose of inactivated SARS-CoV-2 vaccine
Intervention: Biological: inactive SARS-CoV-2 vaccine (Vero cell)
Experimental: Low dose aerosolized Ad5-nCoV group

exercise training group:exercise training plus educational instructions;traditional Chinese medicine group:traditional Chinese medicine plus educational instructions;control group:educational instructions;

Experimental: heterologous boost arm with Ad5 vectored vaccine
Subjects who have been primed with two doses of inactive SARS-CoV-2 vaccine will receive a booster of recombinant SARS-CoV-2 Ad5 vectored vaccine after 3~6 months.
Intervention: Biological: Recombinant SARS-CoV-2 Ad5 vectored vaccine
Active Comparator: homogeneous boost arm with inactive vaccine
Experimental: Experimental Group
200 subjects in group A1 will receive one dose of booster immunization 3 months after the completion of the second dose of primary immunization of the SARS-CoV-2 Inactivated vaccine.
Intervention: Biological: SARS-CoV-2 Inactivated Vaccine
Experimental: Experimental group
Experimental Group (468 subjects) will receive: 1st dose : combined vaccination of COVAX+PPV23; 2nd dose: combined vaccination of COVAX+IIV4
Intervention: Biological: COVAX+PPV23;COVAX+IIV4
Active Comparator: Control group A

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Experimental: JS016 treatment group
Standard therapy + JS016 injection Standard therapy including vital sign monitoring, supplementary oxygen and respiratory support in accordance with the patient's oxygen saturation and respiratory condition.
Intervention: Drug: JS016
Experimental: Experimental Group
600 participants will receive one dose of booster vaccination with high-dosage inactivated SARS-CoV-2 vaccine .
Intervention: Biological: High-dosage SARS-CoV-2 vaccine
Active Comparator: Control Group
Experimental: Population I
3 doses of experimental vaccine (pilot scale batch) were given at day 0, 30 and 60.
Intervention: Biological: Recombinant new coronavirus vaccine (CHO cell) group
Experimental: Population II
3 doses of experimental vaccine (process validation lot 1) at day 0, 30 and 60;

control group:Follow-up and lifestyle guidance;experimental group:rehabilitation training;

Oral leflunomide + inhaled interferon alpha group:Oral leflunomide + inhaled interferon alpha ;inhaled interferon alpha group:inhaled interferon alpha;

Adults (18-59 years old):Subjects were randomly assigned to the low-dose, high-dose, and placebo groups in a ratio of 2:2:1 (the low-dose and high-dose groups were the medium and high-dose groups in the Phase I clinical trial). All subjects received one dose of the trial vaccine or placebo (an excipient without antigen).;Elderly (>=60 years old):Subjects were randomly assigned to the low-dose, high-dose, and placebo groups in a ratio of 2:2:1 (the

A group:Recombinant COVID-19 vaccine (CHO cell);B group:COVID-19 inactivated vaccine (Vero cell);

Experimental Group:FNC+Symptomatic treatment (except antiviral treatment);Control group:Antiviral therapy + symptomatic treatment;

conventional treatment group:conventional treatment;conventional treatment combined with hymecromone tablets group:conventional treatment combined with hymecromone tablets treatment;

Experimental: Population I
One dose of influenza virus split vaccine and one dose of recombinant Novel coronavirus vaccine (CHO cells) were given in the contralateral arm on day 0, the second dose of recombinant Novel coronavirus vaccine (CHO cells) on day 30 and the third dose of recombinant Novel coronavirus vaccine (CHO cells) on day 60, all in the upper arm deltoid

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test group (18-59 years):SARS-CoV-2mRNA vaccine 15ug/0.5ml;control group (18-59 years):COVID-19 Vaccine (Vero Cell), Inactivated;test group (>=60years):SARS-CoV-2mRNA vaccine 15ug/0.5ml;control group (>=60years):COVID-19 Vaccine (Vero Cell), Inactivated;

Experimental: Experimental Group

The experimental group is also called the combined immunization group.260 participants will receive the first dose of COVID-19 vaccine and EV71 vaccine on day 0 and the second dose of COVID-19 vaccine and EV71 vaccine on day 28.

Intervention: Biological: Experimental Group

A1: pGX9501 (0.5 mg)/placebo;A2:pGX9501 (1.0 mg)/placebo;A3:pGX9501 (2.0 mg)/placebo;B1:pGX9501 (0.5 mg)/placebo;B2:pGX9501 (1.0 mg)/placebo;B3:pGX9501 (2.0 mg)/placebo;

Lianhua Qingwen Group:Capsules 4 capsules/time, 3 times a day;Basic treatment group:In principle, no drugs and other treatments are used;

Experimental: Inactivated SARS-CoV-2 vaccine Lot 1 of the workshop 2

360 Participants aged 3-17 years(including 52 children aged 3-5 years,154 children aged 6-11 years and 154 adolescents aged 12-17 years) will receive Inactivated SARS-CoV-2 vaccine Lot 1 of the workshop 2 according to 0,28-day immunization schedule.

Intervention: Biological: Inactivated SARS-CoV-2 Vaccine (Vero cell) Lot 1 of the workshop

Experimental: Investigational vaccine group 1

100 tuberculosis patients will receive two doses of standard dosage CoronaVac plus one dose of double dosage CoronaVac at a schedule of 0, 28, 56 days.

Interventions:

Biological: Standard dosage inactivated vaccine

Experimental: Experimental Group 1

96 subjects who completed primary immunization with inactivated COVID-19 vaccine manufactured by Sinovac Research & Development Co.,Ltd. will receive one dose of booster immunization with inactivated COVID-19 vaccine manufactured by Sinovac Research & Development Co.,Ltd.

Experimental: Experimental Group-High-dosage of COVID-19 vaccine (Vero cell), Inactivated

170 participants will receive one dose of booster vaccination with high-dosage of COVID-19 vaccine (Vero cell), Inactivated.

Intervention: Biological: High-dosage of COVID-19 vaccine (Vero cell), Inactivated

Experimental: IBI314

Low/medium/high dose, intravenously, once, on Day 1

Interventions:

Biological: IBI314(low dose)

Biological: IBI314(high dose)

18 to 59 years Group:Administration of the investigational vaccine (5×10^10vp) or placebo;>=60 years Group:Administration of the investigational vaccine (5×10^10vp) or placebo;

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Placebo Comparator: JS026/placebo 30 mg
4 patients will be enrolled in this arm.
Interventions:
Biological: JS026/placebo
Biological: JS016/placebo
Experimental: Probiotics
The intervention group consumes oral probiotics 1 time/day for 7 consecutive days.
Intervention: Dietary Supplement: Probiotics daily
Placebo Comparator: Placebos
The placebo group consumes oral placebo 1 time/day for 7 consecutive days
Experimental: Probiotics-Placebo
1 week of consecutively consuming oral probiotics (1 time/day) followed by SARS-CoV-2 vaccination, then 2 weeks of consecutively consuming oral placebo (1 time/day).
Interventions:
Dietary Supplement: 1 week of probiotics(Week 1)

1: Jinshuibao tablet;
2: Placebo;

Vaccine: Three doses of experimental vaccine at the schedule of day 0, 28, 56;
Placebo: Three doses of placebo at the schedule of day 0, 28, 56;
Vaccine(China cohort, for immunogenicity bridging): Three doses of experimental vaccine at the schedule of day 0, 28, 56
1 (inactivated vaccine group): subjects were injected with 1 dose of inactivated COVID-19 vaccine (intramuscular injection) at the baseline (day 0);
2 (recombinant protein vaccine group): the subjects were injected with 1 dose of recombinant protein vaccine (intramuscular injection) at the baseline (day 0):
Control group: no intervention

Test group: SARS-CoV-2 mRNA vaccine 15ug/0.5ml
Control group: Novel Coronavirus Inactivated Vaccine (Vero Cells)

Intervention group: MBSR
Control group: Routine care

A1:pGX9501 1.0 mg; A2:pGX9501 2.0 mg; A3:placebo;A4:placebo

Experimental: Group A
Subjects in safety cohort will receive one heterologous booster dose of aerosolized Ad5-nCoV
Intervention: Biological: Aerosolized Ad5-nCoV
Experimental: Group B

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Single-dose A
A single dose A of FB2001 or placebo will be administered by intravenous (IV) infusion
Interventions:
Drug: FB2001
Other: FB2001 placebo

No Intervention: Control Group; Experimental: Inactivated Vaccine Group; Experimental: Recombinant subunit protein vaccine: Experimental: mRNA vaccine Group

Experimental: 18~59 yrs. low dosage (20 μg) - SCTV01C VACCINE
20 participants in Phase I and 120 participants in Phase II at the age of 18~59 years old will receive two doses of low dosage (20 μg/0.5mL) SCTV01C VACCINE on Day 0 and Day 28
Intervention: Biological: SCTV01C

Experimental: AZD7442: co-administration of a single dose of 600 mg AZD7442 (300 mg AZD8895 and 300 mg AZD1061) by intravenous (IV) infusion.;
Placebo Comparator: Placebo: co-administration of a single dose of 600mg placebo by intravenous (IV) infusion.;

control group: routine prone positioning
intervention group: prone positioning+nursing exercise intervention

(Experimental group) Respiratory function exercise combined with normal drug treatment group: Respiratory exercise combined with medication
(Control group) Drug treatment group: medical treatement

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Funding	Primary Sponsor	Registration Status (According to First Enrollment Date and Date of First Submission)
Min	Wuhan Jinyintan Hospital (Wuhan Infectious Diseases Hospital)	Retrospective
#NOTREQ	Peking Union Medical College Hospital	Prospective
#NOTREQ	Beijing 302 Hospital	Retrospective
Chongqing Science and Technology of Bureau	Chongqing Public Health Medical Center	Prospective
Chongqing Science and Technology Bureau	Chongqing Public Health Medical Center	Retrospective
#NOTREQ	Shanghai Public Health Clinical Center	Prospective
Beijing University of Chinese Medicine, Dongzhimen	Dongzhimen Hospital Affiliated to Beijing University of Chinese Medicine	Prospective
#NOTREQ	The Second Hospital of Nanjing Medical University	Prospective
#NOTREQ	Guangzhou 8th People's Hospital	Retrospective

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#NOTREQ	Capital Medical University	Prospective
#NOTREQ	Capital Medical University	Prospective
Self-raised	Affiliated Zhongshan Hospital of Dalian University	Retrospective
Shijiazhuang Yiling Pharmaceutical Co.Ltd.	Hebei Yiling Hospital、Renmin Hospital of Wuhan University	Prospective
Government-funded research projects；Shijiazhuang	Hebei Yiling Hospital, Renmin Hospital of Wuhan University	Prospective
Wuhan 1st Hospital	Wuhan 1st Hospital	Prospective
MOST	Beijing hospital of Traditional Chinese medicine; Hubei integrated traditional Chinese and Western Medicine Hospital	Prospective
MOST	Beijing Hospital of Traditional Chinese Medicine affiliated to Capital Medical University/Beijing institute of Traditional Chinese	Prospective
self-financing	1.Xinhua affiliated hospital, Hubei University of Chinese Medicine; 2. Hubei Provincial Hospital of Integrated Chinese and Western	Prospective
self-financing	1. Xinhua affiliated hospital, Hubei University of Chinese Medicine; 2. Hubei Provincial Hospital of Integrated Chinese and Western	Prospective

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Self-financing	1. Xinnua affiliated hospital, Hubei University of Chinese Medicine; 2. Hubei Provincial Hospital of Integrated Chinese and Western	Prospective
self-financing	1. Xinnua affiliated hospital, Hubei University of Chinese Medicine; 2. Hubei Provincial Hospital of Integrated Chinese and Western	Prospective
Self-financing	1. Xinnua Affiliated Hospital, Hubei University of Chinese Medicine; 2. Hubei Provincial Hospital of Integrated Chinese and Western	Prospective
#NOTREQ	Tongji Hospital	Retrospective
#NOTREQ	Tongji Hospital	Retrospective
#NOTREQ	Tongji Hospital	Retrospective
Self financing and application for scientific research funds	The First Hospital of Changsha; The Second Xiangya Hospital of Central South University	Retrospective
Self-financing	Zhejiang Chinese Medical University	Prospective
Self-financing	Zhejiang Chinese Medical University	Prospective
Self-financing	Tongji Hospital, Tongji Medical College, Huazhong University of Science and Technology	Prospective

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self-financing	Zhongnan Hospital of Wuhan University	Prospective
Self-financing	The First Hospital Affiliated to Zhejiang University's Medical School	Prospective
Self-financing	The First Affiliated Hospital, Zhejiang University School of Medicine	Prospective
Science & Technology Department of Sichuan Province	Hospital of Chengdu University of Traditional Chinese Medicine	Retrospective
Government	Renmin Hospital of Wuhan University	Retrospective
funding	Xiangyang 1st People's Hospital	Prospective
#NOTREQ	Tongji Hospital	Retrospective
#NOTREQ	ZhiYong Peng	Prospective
funding	Xiangyang First People's Hospital	Prospective
coordinater	The First Affiliated Hospital of Medical College of Zhejiang University	Prospective

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Tongji Hospital, Tongji Medical College, Huazhong University of	Department of Hematology, Tongji Hospital, Tongji Medical College, Huazhong University of Science and Technology	Retrospective
raise independently	Beijing hospital of Traditional Chinese Medicine	Prospective
National Science and Technology Major Project of China	Hubei Provincial Hospital of TCM	Retrospective
Project of State Administration of Traditional Chinese Medicine of	Hubei Provincial Hospital of TCM	Retrospective
Self-financing	The First Affiliated Hospital of Zhejiang University School of Medicine	Prospective
Harbin Pharmaceutical Group Sanjing Pharmaceutical Holding Co. Ltd	Tongji Hospital, Tongji Medical College, Huazhong University of Science and Technology	Prospective
#NOTREQ	Shanghai Public Health Clinical Center	Prospective
Emergency Project of Science Technology Department of	The First Affiliated Hospital, College of Medicine, Zhejiang University	Retrospective
Science and Technology Project of Novel Coronavirus Pneumonia in	West China Hospital, Sichuan University	Prospective
Emergency Project for 2019-nCoV of Ministry of Science and	Wuhan Pulmonary Hosptial	Prospective

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Department of Anesthesiology	Central Theater General Hospital of PLA	Prospective
Government	The First Hospital of Peking University	Prospective
Sun Yat-Sen University	The Fifth Affiliated Hospital Sun Yat-Sen University	Prospective
Tongji Hospital,Tongji Medical College, Huazhong	Tongji Hospital,Tongji Medical College, Huazhong University of Science and Technology	Retrospective
Department of Science and Technology of Anhui Province	The First Affiliated Hospital of Anhui University of Traditional Chinese Medicine	Prospective
Tongji Hospital	Tongji Hospital, Tongji Medical College of HUST	Prospective
spontaneous	Renmin Hospital of Wuhan University	Prospective
Government	China-Japan friendship hospital	Prospective
self-financing	The Second Affiliated Hospital of Chongqing Medical University	Prospective
Chonqing Medical University	The Second Affiliated Hospital of Chongqing Medical University	Prospective

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Chongqing Municipal Health Committee	The First Affiliated Hospital of Chongqing Medical University	Prospective
Chongqing Municipal Health Committee	The First Affiliated Hospital of Chongqing Medical University	Prospective
National Key R&D Program of China (No.2020YFC0841500);	China Academy of Chinese Medical Sciences; Zhongnan Hospital of Wuhan University	Prospective
Department of science and technology of Anhui Province	The First Affiliated Hospital of University of science and technology of China (Anhui Provincial Hospital)	Retrospective
Self-financing	Zhongnan Hospital of Wuhan University	Retrospective
Taizhou Science and Technology Plan Project	Enze Hospital of Taizhou Enze Medical Center (Group)	Prospective
self-raised	The First Affiliated of Wenzhou Medical University	Retrospective
Emergency Committee of the World Federation of Chinese	Huangshi Hospital of Traditional Chinese Medicine	Retrospective
Lizhu Group Limin Pharmaceutical Factory	Union Hospital affiliated to Tongji Medical College of Huazhong University of Science and Technology	Prospective
Sichuan Guangda Pharmacy Co., Ltd.	Union Hospital affiliated to Tongji Medical College of Huazhong University of Science and Technology	Prospective

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Horizontal project support	Dongzhimen Hospital Affiliated to Beijing University of Chinese Medicine	Prospective
Central Plains Scholar Scientist Studio Project	The First Affiliated Hospital of He'nan University of Chinese Medicine	Prospective
Tongji University	Shanghai Pulmonary Hospital	Prospective
#NOTREQ	Xinxiang medical university	Prospective
self-raised	Renmin Hospital of Wuhan University	Prospective
Ministry of Science and Technology of China	Wuhan Jinyintan Hospital (Wuhan Infectious Diseases Hospital)	Retrospective
Shangnai Municipal Development & Reform Commission	Shanghai Public Health Clinical Center	Prospective
self	Nanjing Second Hospital	Retrospective
Guangznou Regenerative Medicine and Health Guangdong	Jingzhou Central Hospital	Prospective
#NOTREQ	Wuhan Union Hospital, China	Prospective

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#NOTREQ	Second Affiliated Hospital of Wenzhou Medical University	Prospective
Guangznou Regenerative Medicine and Health Guangdong	Jingzhou Central Hospital	Prospective
Science and Technology Department of He'nan Province	The First Affiliated Hospital of Zhengzhou University	Retrospective
Fudan University	Zhongshan Hospital, Fudan University	Prospective
self-raised	People's Hospital of Guangshan County	Prospective
raise independently	The First Affiliated Hospital of Medical College of Zhejiang University	Prospective
Ministry of Science and Technology of the People's Republic of	Beijing You'an Hospital, Capital Medical University	Prospective
SPH SHANGHAI ZHONGXI PHARMACEUTICAL CO., LTD	Ruijin Hospital, Shanghai Jiaotong University School of Medicine	Retrospective
Emergency Committee of the World Federation of Chinese	Huangshi Hospital of Traditional Chinese Medicine	Retrospective
#NOTREQ	Beijing Chao Yang Hospital	Retrospective

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#NOTREQ	ShuGuang Hospital	Prospective
Peking University Third Hospital self-funds	Peking University Third Hospital	Prospective
Peking University Third Hospital self-fund	Peking University Third Hospital	Prospective
HwaMei Key Research Fund (2020HMZD18)	HwaMei Hospital, University of Chinese Academy of Sciences	Retrospective
Ministry of Science and Technology, China；Fund of Shanghai	Longhua Hospital Affiliated to Shanghai University of Traditional Chinese Medicine	Prospective
Ministry of Science and Technology, China; Fund of Shanghai	Longhua Hospital Affiliated to Shanghai University of Traditional Chinese Medicine	Prospective
spontaneous	Hubei Hospital of Traditional Chinese Medicine	Prospective
Science & Technology Department of Sichuan Province	Hospital of Chengdu University of Traditional Chinese Medicine	Prospective
Self-financing	Tongji Hospital, Tongji Medical College,Huazhong University of Science and Technology	Prospective
Health Commission of Shandong Province, Department of	Qingdao East Sea Pharmaceutical Co., Ltd.	Retrospective

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Emergency Research Project of Shanghai University of	Yueyang Hospital of Integrated Traditional Chinese and Western Medicine, Shanghai University of Traditional	Prospective
Hubei Clinical Research Center for Emergency and Resuscitation	Zhongnan Hospital of Wuhan University	Retrospective
Hospital emergency research project	Chinese PLA General Hospital	Prospective
The National Key Research and Development Program of	Institute of Basic Medical Sciences, Chinese Academy of Medical Sciences	Retrospective
Self-financing	The First Affiliated Hospital of Nanchang University	Prospective
Xiamen science and Technology Bureau	Zhongshan Hospital Affiliated to Xiamen University	Retrospective
Shijiaznuang Pharmaceutical Group Co., Ltd./ Lei Yunshang Pharmaceutical	The First Affiliated Hospital of Guangzhou Medical University	Prospective
Shanghai University of TCM research program for 2019-ncov	Shanghai University of TCM	Prospective
Health commission of Heilongjiang province	Health commission of Heilongjiang province	Retrospective
Anhui Development and Reform Commission	The First Affiliated Hospital of University of science and technology of China (Anhui Provincial Hospital)	Retrospective

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Science & Technology Department of Sichuan Province	Hospital of Chengdu University of Traditional Chinese Medicine	Prospective
National Key Research and Development Project (2018YFC2001	Union Hospital, Tongji Medical College, Huazhong University of Science and Technology	Prospective
Self financing	The First Affiliated Hospital of Guangzhou Medical University	Retrospective
Sinopharm Wuhan Blood Products Co., Ltd.	Wuhan Jinyintan Hospital (Wuhan Infectious Diseases Hospital)	Prospective
Evaluation of efficacy of mycobacterium vacce for injection for	Guangxi medical uniwresity	Retrospective
#NOTREQ	China Academy of Chinese Medical Sciences	Prospective
Self-raised	Beijing Chaoyang Hospital, Capital Medical University	Prospective
Self-funding	Wuhan Fourth Hospital	Retrospective
Self-financing	First Affiliated Hospital of Anhui Medical University	Prospective
Self-financing	West China Hospital	Prospective

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Guangznou Regenerative Medicine and Health Guangdong	The Sixth Affiliated Hospital of Guangzhou Medical University (Qingyuan People's Hospital)	Retrospective
The project of the First Affiliated Hospital of Guangzhou	The First Affiliated Hospital of Guangzhou Medical University	Prospective
Science Technology Department of Zhejiang Province	Tongde Hospital of Zhejiang Province	Retrospective
Researcher self-financing	Peking University Third Hospital	Prospective
#NOTREQ	The First Affiliated Hospital of Guangzhou Medical University	Retrospective
Xiamen science and Technology Bureau	Zhongshan Hospital Affiliated to Xiamen University	Prospective
Major regional immunity cultivation project, 91742109	The First Affiliated Hospital of Guangzhou Medical University	Retrospective
Science and technology department of Hubei province	Renmin Hospital of Wuhan University	Prospective
Government	The First Affiliated Hospital of Nanchang University	Prospective
The First Hospital of Shanxi Medical University (136 medical	The First Hospital of Shanxi Medical University	Prospective

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Funding for emergency research projects	The Sixth Medical Center of PLA General Hospital	Prospective
Shangnai Changzheng Hospital Project Fund and Drug donation from	Shanghai Changzheng Hospital	Prospective
self-financing	Tianjin Huanhu Hospital	Prospective
Financial Appropriation of Jiangxi Provincial Department of	The First Affiliated Hospital of Nanchang University	Retrospective
Self-financing	Shanghai Public Health Clinical Center	Retrospective
Shenzhen Science and Technology Bureau	Shenzhen national medicine inheritance medical research institute co. LTD	Prospective
self-financing	The Second Affiliated Hospital Of Xi'an Jiaotong University	Prospective
Chinese PLA General Hospital	Chinese PLA General Hospital	Prospective
COVID-19 emergency research project of Chinese PLA general hospital	The 5th Medical Center Chinese PLA General Hospital	Prospective
Self-funded	Renmin Hospital of Wuhan University	Prospective

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Government support and enterprise self-financing	Hunan yuanpin Cell Biotechnology Co., Ltd	Retrospective
Raise independently	The First Affiliated Hospital of Nanchang University	Prospective
Self financing	Jingzhou First People's Hospital	Prospective
自筹	Guangdong Provincial Hospital of Chinese Medicine - Zhuhai Hospital	Retrospective
#NOTREQ	Beijing 302 Hospital	Prospective
#NOTREQ	Puren Hospital Affiliated to Wuhan University of Science and Technology	Prospective
Scientific research project on new coronavirus pneumonia in	West China Hospital of Sichuan University	Retrospective
Not stated	The First Affiliated Hospital of Nanchang University, Ten layers pulse Research Institute of traditional Chinese medicine	Prospective
Ganzhou Science and Technology Bureau	Fifth People's Hospital of Ganzhou	Retrospective
other	Union Hospital, Tongji Medical College, Huazhong University of Science and Technology	Prospective

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Self financing	Yueyang Hospital of Integrated Traditional Chinese and Western Medicine, Shanghai University of Traditional	Retrospective
China National Center for Biotechnology Development	Zhongnan Hospital of Wuhan University	Retrospective
SPH Xing Ling Sci. & Tech. Pharmaceutical Co.,Ltd	Shanghai University of Traditional Chinese Medicine	Prospective
Ascletis Pharmaceuticals CO., LTD.	Shanghai Changzheng Hospital	Retrospective
Raise independently	Ganzi Hospital of West China Hospital, Sichuan University	Prospective
Wuxi novel coronavirus infection emergency control	Wuxi Fifth People's Hospital	Prospective
Novel coronavirus pneumonia clinical optimization	The First Affiliated Hospital of Harbin Medical University	Retrospective
National Key R&D Program of China (No.2020YFC0841500);	China Academy of Chinese Medical Sciences	Prospective
Tongji Hospital of Tongji Medical College; Huazhong	Tongji Hospital of Tongji Medical College; Huazhong Science and Technology University	Prospective
Project leader's SCI funds	West China Hospital of Sichuan University	Prospective

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NO	Union Hospital, Tongji Medical College, Huazhong University of Science & Technology	Prospective
Xi'an Medical university	The Second Affiliated Hospital of Xi'an Medical University	Prospective
#NOTREQ	Ansun Biopharma, Inc.	Prospective
Sinopharm Wuhan Blood Products Co., Ltd.	First people's hospital of Jiangxi district，Wuhan	Prospective
raise independently	Jingzhou First People's Hospital	Retrospective
Government, foundation, company	Wuhan Jinyintan Hospital (Wuhan Infectious Diseases Hospital)	Retrospective
raise independently	Harbin infectious diseases hospital	Prospective
self-finance	The First Affiliated Hospital of Fujian Medical University	Prospective
Liaoning province science and technology plan project	Institute of Integrative Medicine of Dalian Medical University, Dalian, China.	Prospective
Optional subjects	The First Affiliated Hospital of Fujian Medical University	Prospective

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Department of Anesthesiology	The First Affiliated Hospital of Guangzhou University of TCM	Prospective
Liaoning province science and technology plan project	Dalian Medical University, Dalian, China.	Retrospective
Shanghai University of T.C.M.	Shanghai University of TCM	Retrospective
Self-funded	Maoming People's Hospital	Prospective
Prevention and treatment of novel coronavirus by Shanghai	Cancer Institute, Longhua Hospital, Shanghai University of Traditional Chinese Medicine	Prospective
Self-financing	Affiliated Hospital of traditional Chinese and Western Medicine Nanjing University of Chinese Medicine	Retrospective
Self-financing	Affiliated Hospital of traditional Chinese and Western Medicine Nanjing University of Chinese Medicine	Retrospective
Tongji Hospital of Tongji Medical College, Huazhong	Department of Respiratory and Critical Care Medicine, Tongji Hospital of Tongji Medical College, Huazhong University of Science and	Prospective
Science and Technology Department of Hubei Province	Zhongnan Hospital of Wuhan University	Retrospective
-	The first affiliated hospital of guangzhou university of Chinese medicine	Retrospective

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Beijing Darwin Cell Biotechnology Co., Ltd	Hubei Shiyan Taihe hospital	Retrospective
#NOTREQ	Henan Provincial People's Hospital	Prospective
self-financing	The First Hospital of Harbin Medical University	Prospective
Self-financing	The Second Affiliated Hospital of Wenzhou Medical University	Retrospective
Project of Sichuan Provincial Department of Science and	Affiliated Hospital of Chengdu University of Traditional Chinese Medicine	Prospective
self-funding	Wuhan Red Cross Hospital	Retrospective
Huaznong University of Science and Technology COVID-19	Tongji Hospital Affiliated to Tongji Medical College of Huazhong University of science and technology	Prospective
spontaneous	Hubei Hospital of Traditional Chinese Medicine	Retrospective
self-raising	Shandong Provincial Hospital	Retrospective
supported by Zhejiang University special scientific	Shanghai General Hospital	Retrospective

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Science and Technology Department of He'nan Province	The First Affiliated Hospital of Zhengzhou University	Retrospective
#NOTREQ	Peking University First Hospital	Retrospective
The investigator will personally pay the additional costs of normal	Liaocheng People's Hospital	Prospective
#NOTREQ	Qilu Hospital of Shandong University	Prospective
Self-raised	Union Hospital affiliated to Tongji Medical College of Huazhong University of Science and Technology	Retrospective
Self-raised	The Third People's Hospital of Shenzhen	Prospective
Government	China-Japan friendship hospital	Retrospective
National project	Xiangya Hospital of Central South University	Prospective
Raise independently	The First People's Hospital of Jiangxia District, Wuhan	Retrospective
Hubei Clinical Research Center for Emergency and Resuscitation	Zhongnan Hospital of Wuhan University	Retrospective

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National Science and Technology Major Project of China	Hubei Provincial Hospital of TCM	Retrospective
Apply for special fund support	Shenzhen Third People's Hospital	Retrospective
Wenzhou science and Technology Bureau	The First Affiliated Hospital of Wenzhou Medical University	Retrospective
Shangnai Changzheng Hospital Project Fund and Drug donation from	Shanghai Changzheng Hospital	Prospective
Emergent Technology Project of Maoming	Maoming People's Hospital	Retrospective
#NOTREQ	Zhong Wang	Prospective
Beijing continent Pharmaceutical Company Limited, 120000	State Key Laboratory of Respiratory Disease, National Clinical Center for Respiratory Disease, Guangzhou Institute of	Retrospective
2020YJY0202	HwaMei Hospital, University of Chinese Academy of Sciences	Retrospective
2020YJY0201	HwaMei Hospital, University of Chinese Academy of Sciences	Retrospective
Self-financing	Foshan First People's Hospital	Retrospective

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Department of science and technology of Shaanxi Province	Affiliated Hospital of Shaanxi University of Traditional Chinese Medicine	Retrospective
Sinopharm Wuhan Blood Products Co., Ltd.	Renmin Hospital of Wuhan University	Prospective
Joint project of Science and Technology Department of Jiangxi	The First Affiliated Hospital of Nanchang University	Prospective
Guangznou Baiyunshan Jingxiutang Pharmaceutical Co., Ltd	The First Affiliated Hospital of Guangzhou Medical University	Prospective
Funded by the Science and Technology departments of Jiangxi	The Second Affiliated Hospital of Nanchang University	Prospective
#NOTREQ	Beijing Chao Yang Hospital	Retrospective
National key R & D plan 2020YFC0841500	Guangdong Provincial Hospital of Chinese Medicine	Prospective
Basic scientific research business expenses of central level	Xiyuan Hospital, Chinese Academy of Traditional Chinese Medicine	Retrospective
Funding of Brand Subjects of Ningbo Kangning Hospital	Ningbo Kangning Hospital	Prospective
Science and Technology Project Fund of Changsha Science and	The third xiangya hospital of Central South University	Retrospective

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Shanghai Key Discipline Construction-Infectious Diseases	Shanghai Public Health Clinical Center	Prospective
#NOTREQ	Guangzhou Institute of Respiratory Disease	Prospective
The National Science and Technology Major Projects for	Beijing you'an Hospital, Capital Medical University	Retrospective
#NOTREQ	Shanghai Asclepius Meditec Inc.	Retrospective
#NOTREQ	Chongqing Public Health Medical Center	Retrospective
#NOTREQ	Fuzhou General Hospital	Retrospective
Three major scientific research projects of Sun Yat-sen	The Third Affiliated Hospital of Sun Yat-sen University	Prospective
The Pioneering Program of the First Teaching Hospital of Tianjin	First Teaching Hospital of Tianjin University of Traditional Chinese Medicine	Retrospective
#NOTREQ	Renmin Hospital of Wuhan University	Prospective
Tongji Hospital of Tongji Medical College, Huazhong	Tongji Hospital of Tongji Medical College, Huazhong Science and Technology University	Prospective

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government	Infection prevention and treatment center of The Fifth Affiliated Hospital of Sun Yat-Sen University	Retrospective
#NOTREQ	Peking University First Hospital	Prospective
#NOTREQ	Peking University First Hospital	Prospective
Shanghai sixth people's hospital	Shanghai Sixth People's Hospital	Retrospective
Novel coronavirus pneumonia emergency project of	Beijing University of Chinese Medicine	Retrospective
#NOTREQ	Institute of Biotechnology, Academy of Military Medical Sciences, PLA of China	Prospective
Huzhou science and technology bureau	Huzhou Central Hospital	Retrospective
Ministry of Science and Technology, China	Henan Provincial Center for Disease Control and Prevention	Prospective
Jiangsu CF PharmTech, Inc.	Jiangsu Province Hospital	Prospective
self-finance	Guangzhou Eighth People's Hospital	Retrospective

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#NOTREQ	Sinovac Research and Development Co., Ltd.	Prospective
National Key R&D Program of China (No.2020YFC0841500)	China Academy of Chinese Medical Sciences	Prospective
emergency special project of the Science and Technology Ministry in	Beijing University of traditional Chinese Medicine	Retrospective
Logistics research Project	Sixth Medical Center of PLA General Hospital	Prospective
Shanghai Public Health Clinical Center	Shanghai Public Health Clinical Center	Prospective
#NOTREQ	Guang'anmen Hospital of China Academy of Chinese Medical Sciences	Prospective
National Key Research and Development Project	Hubei Provincial Hospital of TCM	Prospective
Science and Technology Department of State Administration	Wuhan Hospital Of Traditional Chinese and Western Medicine (Wuhan 1st Hospital)	Retrospective
Ministry of science and technology	Hubei Provincial Hospital of TCM	Prospective
Novel coronavirus pneumonia clinical optimization	Mudanjiang Kang'an Hospital	Prospective

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#NOTREQ	Xijing Hospital	Retrospective
Self-financing	Beijing University of Chinese Medicine Third Affiliated Hospital	Prospective
West China Hospital, Sichuan University	West China Hospital, Sichuan University	Retrospective
Anti-epidemic emergency research project of Shanghai University of	Yueyang Integrated Traditional Chinese and Western Medicine Hospital Affiliated to Shanghai University of Traditional	Retrospective
Ministry of science and technology	Xiaogan Traditional Chinese Medicine Hospital	Retrospective
None	West Campus of Union Hospital affiliated to Tongji Medical College of Huazhong University of Science and Technology	Prospective
Ministry of Science and Technology, China	Henan Provincial Center for Disease Control and Prevention	Prospective
Research funds	Shanghai Public Health Clinical Center	Retrospective
National major research and development plan, Key project of	State Key Laboratory of Respiratory Disease, Guangzhou Institute of Respiratory Diseases, the First Affiliated Hospital of	Prospective
National Science and Technology Major Project of China	Hubei Provincial Hospital of TCM	Prospective

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Special Research Project on Prevention and Control of	The First Affiliated Hospital of Guangdong Pharmaceutical University; Guangzhou Eighth People's Hospital	Prospective
The sponsors raise the money themselves	Beijing Ditan Hospital affiliated to Capital Medical University	Prospective
#NOTREQ	Sinovac Research and Development Co., Ltd.	Prospective
#NOTREQ	Chinese Medical Association	Retrospective
Institute of Science and Technology, Guangzhou University of	Hongqi Hospital Affiliated to Mudangjiang Medical University	Retrospective
cience and Technology Department of State Administration	Wuhan Third Hospital	Retrospective
China Medical University	The First Hospital of China Medical University	Retrospective
Harbin Pharmaceutical Group Sanjing Pharmaceutical Holding Co. Ltd	The First Affiliated Hospital of Wenzhou Medical University	Retrospective
#NOTREQ	Chinese Academy of Medical Sciences	Retrospective
No funding.	Jiangsu People's Hospital	Retrospective

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Self-financing	Guangzhou Women and Children's Medical Center	Prospective
self-raised	Xiangyang Central Hospital	Retrospective
Anti-epidemic emergency research project of Shanghai University of	Yueyang Integrated Traditional Chinese and Western Medicine Hospital Affiliated to Shanghai University of Traditional	Retrospective
Shanghai Science and Technology Commission	Department of Pulmonology, Shuguang Hospital Affiliated to Shanghai University of Traditional Chinese Medicine	Retrospective
#NOTREQ	Shanghai Junshi Bioscience Co., Ltd.	Retrospective
Wanbangde Pharmaceutical Group Co.,Ltd.	Children's Hospital Affiliated to Zhejiang University School of Medicine	Retrospective
#NOTREQ	Anhui Zhifei Longcom Biologic Pharmacy Co., Ltd.	Prospective
Bengbu Medical College	Bengbu Medical College	Retrospective
Self-funded	Shulan (Hangzhou) Hospital; Center for Disease Control and Prevention of Guangxi Zhuang Autonomous Region	Prospective
#NOTREQ	Anhui Zhifei Longcom Biologic Pharmacy Co., Ltd.	Prospective

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#NOTREQ	Chinese Academy of Medical Sciences	Retrospective
#NOTREQ	Brii Biosciences Limited	Retrospective
#NOTREQ	Brii Biosciences Limited	Retrospective
#NOTREQ	Sinocelltech Ltd.	Prospective
Wuhan Municipal Health Commission	Hubei Hospital of Traditional Chinese Medicine	Retrospective
Shanghai Public Health Clinical Center	Shanghai Public Health Clinical Center	Prospective
#NOTREQ	Anhui Zhifei Longcom Biologic Pharmacy Co., Ltd.	Prospective
#NOTREQ	BioNTech SE	Retrospective
#NOTREQ	Mabwell (Shanghai) Bioscience Co., Ltd.	Retrospective
#NOTREQ	Jiangsu Province Centers for Disease Control and Prevention	Prospective

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Innovation Fund for Medical Sciences sponsored by Chinese	Chinese Academy of Medical Sciences	Prospective
Self financing	Jiangsu Provincial Center For Disease Control and Prevention (Public Health Research Institute of Jiangsu Province)	Prospective
Sponosor Investment	Huashan Hospital Affiliated to Fudan University	Prospective
#NOTREQ	Sinovac Research and Development Co., Ltd.	Prospective
Cross-topic of special emergency for pneumonia prevention and	The Third Affiliated Hospital of Beijing University of traditional Chinese Medicine	Prospective
Beijing University of Chinese Medicine 2020 Basic Scientific	The Third Affiliated Hospital of Beijing University of traditional Chinese Medicine	Prospective
#NOTREQ	CanSino Biologics Inc.	Prospective
#NOTREQ	Shanghai Junshi Bioscience Co., Ltd.	Prospective
Beijing University of Traditional Chinese Medicine 2020	Beijing University of Chinese Medicine Third Affiliated Hospital	Retrospective
Self-funded	Jiangsu Provincial Center for Disease Control and Prevention(Public Health Research Institute of Jiangsu Province)	Prospective

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#NOTREQ	Guangzhou Institute of Respiratory Disease	Prospective
Self-funded	Shulan (Hangzhou) Hospital	Prospective
Xiangzhou District People's Hospital	Xiangyang Central Hospital	Retrospective
Self-funded	Jiangsu Provincial Center for Disease Control and Prevention (Public Health Research Institute of Jiangsu Province)	Retrospective
#NOTREQ	Sinovac Research and Development Co., Ltd.	Retrospective
Self financing	Jiangsu Provincial Center For Disease Control and Prevention (Public Health Research Institute of Jiangsu Province)	Prospective
#NOTREQ	Mabwell (Shanghai) Bioscience Co., Ltd.	Prospective
#NOTREQ	BioNTech SE	Prospective
#NOTREQ	Jiangsu Province Centers for Disease Control and Prevention	Retrospective
#NOTREQ	Jiangsu Province Centers for Disease Control and Prevention	Retrospective

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Sponosor	Jiangsu Provincial Center for Disease Control and Prevention	Prospective
Self-funded	Hunan Provincial Center for Disease Control and Prevention	Prospective
#NOTREQ	Xi'an International Medical Center Hospital	Retrospective
Shanghai Science and Technology Commission	Shanghai Public Health Clinical Center	Prospective
#NOTREQ	Brii Biosciences Limited	Prospective
Project funded by Shanxi Provincial Key Laboratory for major infectious	Shanxi Provincial Center for Disease Control and Prevention	Retrospective
Project funded by Shanxi Provincial Key Laboratory for major infectious	Shanxi Provincial Center for Disease Control and Prevention	Prospective
Project funded by Shanxi Provincial Key Laboratory for major infectious	Shanxi Center for Disease Control and Prevention	Prospective
Self-funded	Yunnan Walvax Biotechnology Co., Ltd.	Prospective
Shijiazhuang Yiling Pharmaceutical Co., Ltd.	Hebei Province Chest Hospital	Retrospective

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Shijiazhuang Yiling Pharmaceutical Co., Ltd.	Hebei Province Chest Hospital	Retrospective
Shijiazhuang Yiling Pharmaceutical Co., Ltd.	Hebei Province Chest Hospital	Retrospective
self-financing	Shu Lan (Hang Zhou) Hospital	Prospective
#NOTREQ	Jiangsu Provincial Center for Disease Control and Prevention	Prospective
covid-19 Emergency project	Tianjin University of TCM	Prospective
Shijiazhuang Yiling Pharmaceutical Co.Ltd.	The Second Hospital of Hebei Medical University	Prospective
Sichuan Jishengtang Pharmaceutical Co. Ltd	Hubei Hospital of Traditional Chinese Medicine	Retrospective
#NOTREQ	Brii Biosciences Limited	Prospective
Translational Medical Center for Stem Cell Therapy of Shanghai	Shanghai Oriental Hospital	Retrospective
#NOTREQ	Sinovac Research and Development Co., Ltd.	Prospective

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#NOTREQ	Jiangsu Province Centers for Disease Control and Prevention	Prospective
None	Public Health Clinical Center of Chengdu	Prospective
#NOTREQ	Jiangsu Province Centers for Disease Control and Prevention	Prospective
Livzon Mabpharm Inc.	Guangdong Provincial Center for Disease Control and Prevention/ Gaozhou Municipal Center for Disease Control and Prevention	Retrospective
Livzon Mabpharm Inc.	Guangdong Provincial Center for Disease Control and Prevention/ Gaozhou Municipal Center for Disease Control and Prevention	Retrospective
Self-funded	The First Affiliated Hospital, Sun Yat-Sen University	Prospective
#NOTREQ	CanSino Biologics Inc.	Prospective
#NOTREQ	National Vaccine and Serum Institute, China	Prospective
#NOTREQ	China National Biotec Group Company Limited	Prospective
Shangnai Municipal Science and Technology Commission	shulan (Hangzhou) Hospital	Retrospective

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#NOTREQ	Sinovac Research and Development Co., Ltd.	Retrospective
China National Biotec Group Company Limited; The National Key	Shanghai Municipal Center for Disease Control and Prevention	Prospective
Wuhan Institute of Biological Products Co., Ltd.	Hubei Provincial Center for Disease Control and Prevention	Prospective
Project of Ministry of science and technology	Shijiazhuang People's Hospital	Retrospective
#NOTREQ	Qiang Hu	Retrospective
#NOTREQ	CanSino Biologics Inc.	Prospective
#NOTREQ	Jiangsu Province Centers for Disease Control and Prevention	Prospective
Self financing	Jiangsu Provincial Center For Disease Control and Prevention (Public Health Research Institute of Jiangsu Province)	Retrospective
Translational Medical Center for Stem Cell Therapy of Shanghai	Shanghai East Hospital	Retrospective
Guangdong Provincial Department of Science and Technology	Guangdong Provincial Hospital of Traditional Chinese Medicine	Retrospective

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#NOTREQ	Zhejiang Clover Biopharmaceuticals, Inc.	Prospective
#NOTREQ	Luye Pharma Group Ltd.	Retrospective
#NOTREQ	Sinovac Research and Development Co., Ltd.	Prospective
#NOTREQ	Anhui Zhifei Longcom Biologic Pharmacy Co., Ltd.	Retrospective
#NOTREQ	Shanghai Zerun Biotechnology Co.,Ltd	Retrospective
#NOTREQ	Shanghai Zerun Biotechnology Co.,Ltd	Retrospective
#NOTREQ	Sinovac Research and Development Co., Ltd.	Prospective
#NOTREQ	Shenzhen Kangtai Biological Products Co., LTD	Prospective
#NOTREQ	Shenzhen Kangtai Biological Products Co., LTD	Retrospective
#NOTREQ	Sinovac Research and Development Co., Ltd.	Prospective

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#NOTREQ	WestVac Biopharma Co., Ltd.	Retrospective
Ningbo Rong an Biological pharmaceutical Co., LTD	Hunan Provincial Center for Disease Control and Prevention	Prospective
Jiangsu Provincial Department of Science and Technology	Affiliated Hospital of Nanjing University of Chinese Medicine	Retrospective
State Administration of Traditional Chinese Medicine	State Administration of Traditional Chinese Medicine	Retrospective
National Administration of Traditional Chinese Medicine	Guangdong Provincial Hospital of Chinese Medicine	Retrospective
National Key R&D Program of China (No.2020YFC0841500);	China Academy of Chinese Medical Sciences; Jiangsu Privince Hospital of Chinese Medicine; Yangzhou Hospital of Traditional	Prospective
Self-funded	Walvax Biotechnology Co., Ltd.	Prospective
Self-funded	Shanxi Provincial Center for Disease Control and Prevention	Retrospective
Chongqing Key Laboratory of Ophthalmology (CSTC, 2008CA5003)	The First Affiliated Hospital of Chongqing Medical University	Prospective
Self-funded	Shulan (Hangzhou) Hospital	Retrospective

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Self-funded	Guangxi Center for Disease Control and Prevention	Prospective
Health Commission of Hubei Province	Hubei Provincial Hospital of Integrated Chinese and Western Medicine	Retrospective
#NOTREQ	Jieming QU	Prospective
#NOTREQ	Shanghai Public Health Clinical Center	Prospective
Shanghai University of TCM research program for 2019-ncov	Shanghai University of TCM	Prospective
#NOTREQ	Jiangsu Province Centers for Disease Control and Prevention	Prospective
Health Commission of Hubei Province	Hubei Provincial Hospital of Integrated Chinese and Western Medicine	Prospective
#NOTREQ	Jiangsu Province Centers for Disease Control and Prevention	Prospective
#NOTREQ	Sinovac Research and Development Co., Ltd.	Retrospective
#NOTREQ	China National Biotec Group Company Limited	Retrospective

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#NOTREQ	Peking Union Medical College Hospital	Retrospective
#NOTREQ	Sinovac Biotech Co., Ltd	Prospective
#NOTREQ	Anhui Zhifei Longcom Biologic Pharmacy Co., Ltd.	Retrospective
Hubei Provincial Health and Health Commission	Taihe Hospital of Shiyan City	Prospective
self-raised	Renmin Hospital of Wuhan University	Prospective
Ningbo Rong 'an biological pharmaceutical Co. LTD	Hunan Provincial Center for Disease Control and Prevention	Prospective
Chen Zhihai's funding for Changjiang Scholars	Beijing Ditan Hospital, Capital Medical University	Retrospective
The First Affiliated Hospital of Zhengzhou University	The First Affiliated Hospital of Zhengzhou University	Retrospective
Study group at their own expense	Shanghai Public Health Clinical Center	Retrospective
#NOTREQ	Anhui Zhifei Longcom Biologic Pharmacy Co., Ltd.	Retrospective

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Self-funded	Walvax Biotechnology Co., Ltd.	Prospective
#NOTREQ	Sinovac Biotech Co., Ltd	Prospective
sponsor	Sichuan Provincial Center for Disease Control and Prevention	Prospective
Sponsor	Shanghai Public Health Clinical Center	Retrospective
#NOTREQ	Sinovac Research and Development Co., Ltd.	Retrospective
#NOTREQ	Jiangsu Province Centers for Disease Control and Prevention	Prospective
#NOTREQ	Sinovac Research and Development Co., Ltd.	Prospective
#NOTREQ	Sinovac Research and Development Co., Ltd.	Retrospective
#NOTREQ	Innovent Biologics (Suzhou) Co. Ltd.	Prospective
Self-funded	Yunnan Walvax Biotechnology Co., Ltd.	Prospective

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#NOTREQ	Shanghai Junshi Bioscience Co., Ltd.	Prospective
#NOTREQ	Peking University	Prospective
#NOTREQ	Peking University	Prospective
NA	Hubei Provincial Hospital of Traditional Chinese Medicine	Retrospective
Self-funded	Sichuan Center for Disease Control and Prevention	Retrospective
Beijing Ditan Hospital, Capital Medical University	Beijing Ditan Hospital, Capital Medical University	Retrospective
Self-funded	Yunnan Walvax Biotechnology Co., Ltd.	Retrospective
Postgraduate Research & Practice Innovation Program of	Yangzhou University	Retrospective
Sponosor	Jiangsu Provincial Center for Disease Control and Prevention	Prospective
#NOTREQ	Jiangsu Province Centers for Disease Control and Prevention	Prospective

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#NOTREQ	Frontier Biotechnologies Inc.	Prospective
#NOTREQ	Huashan Hospital	Retrospective
#NOTREQ	Sinocelltech Ltd.	Prospective
#NOTREQ	AstraZeneca	Prospective
Sichuan Health Care Commission Medical Major Innovation	West China Hospital of Sichuan University	Retrospective
Changde City Science and Technology Bure	The first people's hospital of changde city	Retrospective

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Ethical Approval Status (Approved = 1 or 0 = 0)	Name of Ethic Committee	Ethic Committee Approval No.
1	Medical Ethics Committee of Wuhan Infectious Disease Hospital	KY-2020-02.01
#NOTREQ	#NOTREQ	#NOTREQ
#NOTREQ	#NOTREQ	#NOTREQ
1	The Ethic Committee of Chongqing Public Health Medical Center	2020-003-02-KY
1	The Institution Review Board of Chongqing Public Health Medical Center	2020-002-01-KY
#NOTREQ	#NOTREQ	#NOTREQ
1	Ethics Committee of Dongzhimen Hospital Affiliated to Beijing University of Chinese Medicine	DZMEC-KY-2020-09
#NOTREQ	#NOTREQ	#NOTREQ
#NOTREQ	#NOTREQ	#NOTREQ

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#NOTREQ	#NOTREQ	#NOTREQ
#NOTREQ	#NOTREQ	#NOTREQ
1	Zhongshan Hospital Affiliated to Dalian University ethics committee	2020003
1	Ethic committee of Hebei Yiling Hospital	2020LCKY-002
1	Ethics Committee of Hebei Yiling Hospital/ Ethics Committee of Renmin Hospital of Wuhan University	2020LCKY-001/ WDRY2020-K027
1	Medical ethics committee of Wuhan 1st Hospital	武卫一院伦审[2020]1号
1	Clinical trial ethics committee of Hubei integrated hospital of traditional Chinese and Western Medicine	[2020]004
1	Clinical trial ethics committee of Hubei integrated hospital of traditional Chinese and Western Medicine	[2020]002
1	Ethics Committee of Hubei Provincial Hospital of Integrated Chinese and Western Medicine	2020007
1	Ethics Committee of Hubei Provincial Hospital of Integrated Chinese and Western Medicine	2020008

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1	Ethics Committee of Hubei Provincial Hospital of Integrated Chinese and Western Medicine	2020006
1	Ethics Committee of Hubei Provincial Hospital of Integrated Chinese and Western Medicine	2020009
1	Ethics Committee of Hubei Provincial Hospital of Integrated Chinese and Western Medicine	2020010
#NOTREQ	#NOTREQ	#NOTREQ
#NOTREQ	#NOTREQ	#NOTREQ
#NOTREQ	#NOTREQ	#NOTREQ
1	The Second Xiangya Hospital of Central South University Ethics Committee / Medical Ethics Committee of Changsha 1st Hospital	KX2020-002; 2020-003
1	Medical Ethics Committee of Hangzhou Xixi Hospital	2020年（科）伦审第12号
1	Medical Ethics Committee of Hangzhou Xixi Hospital	2020年（科）伦审第13号
1	The Ethic Committee of Tongji Hospital, Tongji Medical College, Huazhong University of Science and Technology	TJ-C20200102

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1	The ethic committee of Zhongnan Hospital of Wuhan University	临研伦【2020011】
1	Clinical Research Ethics Committee of The First Hospital Affiliated to Zhejiang University's Medical School	（2020）IIT会审第（7）号
1	Clinical Research Ethics Committee of the First Affiliated Hospital,College of Medicine,Zhejiang University	(2020)IIT会审第（6）号
1	Sichuan Regional Ethical Review Committee of Traditional Chinese Medicine	2020QKL－004
1	Clinical Research Ethics Committee of Wuhan University Renmin Hospital	WDRY2020-K012
1	EC of Xiangyang 1st People's Hospital	2020GCP008
#NOTREQ	#NOTREQ	#NOTREQ
#NOTREQ	#NOTREQ	#NOTREQ
1	Ethics Committee of Xiangyang First People's Hospital	2020GCP010
1	Clinical Research Ethics Committee of the First Affiliated Hospital of Medical College of Zhejiang University	（2020）IIT快审第（26）号

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1	Medical ethics committee of Department of Hematology, Tongji Hospital, Tongji Medical College, Huazhong University of Science and Technology	TJ-C20200115
1	Medical Ethics Committee of Tianjin University of TCM	TJUTCM-EC20200001
1	Medical Ethics Committee of Hubei Provincial Hospital of TCM	HBZY2020-C01-01
1	Medical Ethics Committee of Hubei Provincial Hospital of TCM	HBZY2020-C01-01
1	The Clinical Research Ethic Committee of the First Affiliated Hospital of Zhejiang University School of Medicine	（2020）IIT会审第（8）号，（2020）IIT快审第（6）号
1	Ethical Committed of Tongji Hospital, Tongji Medical College, Huazhong University of Science and Technology	TJ-C20200113
#NOTREQ	#NOTREQ	#NOTREQ
1	Ethics Committee on Stem Cell of the First Affiliated Hospital of Zhejiang University School of Medicine	[2020]GXB伦审001号
1	Clinical Trial Ethics Committee, West China Hospital of Sichuan University	2020年临床试验（西药）审（10）号
1	Medical Ethics Committee of Wuhan Pulmonary Hospital	武肺伦理(2020)年第（1）号

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1	Ethics Committee in Central Theater General Hospital of PLA	【2020】009-3
1	Peking University First hospital Human Research Ethics committee	2020科研031
1	Medical Ethics Committee of the Fifth Affiliated Hospital Sun Yat-Sen University	中大五院【2020】伦字第（K15-1）号
1	Ethics Committee of Tongji Hospital, Tongji Medical College, Huazhong University of Science and Technology	TJ-C20200116
1	The First Affiliated Hospital of Anhui University of Traditional Chinese Medicine	2020HSAH-01
1	Institutional Review Board, Tongji Hospital, Tongji Medical College, Huazhong University of Science and Technology	TJ-C20200124
1	Ethics Committee of Clinical Research, Renmin Hospital of Wuhan University	WDRY2020-K018
1	The Ethics Review Committee of Institute of Blood Transfusion, Chinese Academy of Medical Sciences	202002
1	Ethic Committee of the Second Affiliated Hospital of Chongqing Medical University	2020年伦审(2)号
1	The Ethic Committee of the Second Affiliated Hospital of Chongqing Medical University	2020年伦审(1)号

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1	Ethics Committee of the First Affiliated Hospital of Chongqing Medical University	2020年科伦药审（20200101）号
1	The First Affiliated Hospital of Chongqing Medical University ethics committee	20200401
1	Ethics Review Committee of Institute of Basic Research in Clinical Medicine, China Academy of Chinese Medical Sciences	P20001/PJ01
1	The First Affiliated Hospital of University of science and technology of China (Anhui Provincial Hospital) Ethics committee	2020-XG（H）-008
1	The Ethics Committee of Zhongnan Hospital of Wuhan University	临研伦[2020016]
1	Hospital Ethics Committee of Enze Hospital of Taizhou Enze Medical Center (Group)	无
1	Ethics Committee in Clinical Research of the First Affiliated of Wenzhou Medical University	2020第（001）号
1	Ethics Committee of Huangshi Hospital of Traditional Chinese Medicine	HSZY-PJ-2020-001-01
1	Medical Ethics Committee Union Hospital affiliated to Tongji Medical College of Huazhong University of Science and Technology	【2020】伦理字（0012）号
1	Medical Ethics Committee Union Hospital affiliated to Tongji Medical College of Huazhong University of Science and Technology	[2020]论审字（012）号

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0	#BLANK	#BLANK
1	Institutional Review Board of the First Affiliated Hospital of He'nan University of Chinese Medicine	2020HL-016
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#NOTREQ	#NOTREQ	#NOTREQ
1	Ethics Committee of Clinical Research, Renmin Hospital of Wuhan University	WDRY2020-K037
1	Medical Ethics Committee of Wuhan Infectious Disease Hospital	KY-2020-10.02
1	Shanghai Public Health Clinical Center Ethics Committee	YJ-2020-E004-01
1	Medical Ethics Committee of Nanjing Second Hospital	2020-LS-ky006
1	Ethic committee of Jingzhou Central Hospital	2020-004-01
#NOTREQ	#NOTREQ	#NOTREQ

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#NOTREQ	#NOTREQ	#NOTREQ
1	Ethic committee of Jingzhou Central Hospital	2020-003-01
1	Ethics Committee of Scientific Research and Clinical Trials of the First Affiliated Hospital of Zhengzhou University	2020-KY-056
1	Ethics Committee of Zhongshan Hospital, Fudan University	B2020-030
1	People's Hospital of Guangshan County EMC	光伦字[2020]001号
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1	Ethics Committee of Beijing Youan Hospital, Capital Medical University	京佑科伦字[2020]017号
1	Ruijin Hospital Ethic Committee, Shanghai Jiaotong University School of Medicine	（2020）临伦审第（29）号/KY2020-29
1	Ethics Committee of Huangshi Hospital of Traditional Chinese Medicine	HSZYYJ-2020-002-01
#NOTREQ	#NOTREQ	#NOTREQ

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#NOTREQ	#NOTREQ	#NOTREQ
1	Peking University Third Hospital Medical Science Research Ethics Committee	（2020）医伦审第（041-05）号
1	Peking University Third Hospital Medical Science Research Ethics Committee	（2020）医伦审第（042-05）号
1	Human Research Ethics Committee of HwaMei Hospital, University of Chinese Academy of Sciences	PJ-NBEY-KY-2020-006-01
0	#BLANK	#BLANK
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1	Sichuan Regional Ethical Review Committee of Traditional Chinese Medicine	2020QKL一005
1	Ethical Committee of Tongji Hospital, Tongji Medical College,Huazhong University of Science and Technology	TJ-C20200127
1	Ethic Committee of Qilu Hospital of Shandong University	(科）伦审第2020 002号

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0	#BLANK	#BLANK
1	Medical Ethics Committee, Zhongnan Hospital of Wuhan University	2020019
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1	Ethical Committee of Beijing You-An Hospital	京佑科伦字[2020]007号
1	IRB of the First Affiliated Hospital of Nanchang University	【2020】临伦审第001号
1	Medical Ethics Committee of Zhongshan Hospital Affiliated to Xiamen University	xmzsyyky伦理第（2020004）号
1	Ethics Committee of the First Affiliated Hospital of Guangzhou Medical University	医研伦审2020第54号
1	Huangshi city hospital of TCM ethic committee, Hubei province	HSZYYJ-2020-003-01
1	Ethics committee of Heilongjiang Provincial hospital	003JJ02
1	The First Affiliated Hospital of University of science and technology of China (Anhui Provincial Hospital) Ethics committee	2020-XG(H)-016

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1	Sichuan Regional Ethical Review Committee of Traditional Chinese Medicine	2020QKL一006
0	#BLANK	#BLANK
1	Clinical Trial Ethics Committee of the First Affiliated Hospital of Guangzhou Medical University	医研伦审2020第48号
1	Medical Ethics Committee of Wuhan Infectious Disease Hospital	KY-2020-17.02
1	Medical ethics committee of the Fourth People's Hospital of Nanning	202002
#NOTREQ	#NOTREQ	#NOTREQ
1	Ethics Committee of Beijing Chaoyang Hospital, Capital Medical University	2020-科-21-1
1	Wuhan Fourth Hospital ethic committee	202001005
1	IRB of the first affiliated hospital of Anhui Medical University	安医一附院伦申-快-PJ2020-03-19
1	China Ethics Committee of Registering Clinical Trials	ChiECRCT20200035

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1	Institutional Review Board of Qingyuan People's Hospital	IRB-2020-002
1	Ethics Committee of the First Affiliated Hospital of Guangzhou Medical University	医研伦审2020第45号
1	Medical ethic committee of Tongde hospital of ZheJiang Province	浙同德伦理审字第【2020】001号
1	Peking University Third Hospital Medical Science Research Ethics Committee	（2020）医伦审第（033-03）号
#NOTREQ	#NOTREQ	#NOTREQ
1	Medical ethics committee of Zhongshan Hospital Affiliated to Xiamen University	xmzsyyky伦理第（2020004）号
1	Medical Ethics Committee of the First Affiliated Hospital of Guangzhou Medical University	医研伦审2020第55号
1	Clinical research ethics committee of Renmin Hospital of Wuhan University	WDRY2020-K047
1	Clinical Research Ethics Committee of the First Affiliated Hospital of Nanchang University	ndyfy(2020)医研伦审第（40）号
1	Ethic Oommittee of the First Hospital of Shanxi Medical University	Q02

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1	Medical ethics committee of PLA Navy General Hospital	[2018]0830
1	Biomedical research ethics committee of Shanghai Changzheng Hospital	2020SL004
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1	Ethics Committee of Stem Cell Clinical Research Institute of the First Affiliated Hospital of Nanchang University	[2020]06号
1	Medical Ethics Committee of Shanghai Public Health Clinical Center	YJ-2020-E007-01
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For peer review only

1	Ethics Committee of Nanhua Hospital Affiliated to Nanhua University	2020-001-01
1	Medical Research Ethics Committee of the First Affiliated Hospital of Nanchang University	(2020)医研伦审第（41）号
1	Medical Ethics Committee of Jingzhou First People's Hospital	Y20200209
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1	Biomedical Ethics Committee of West China Hospital, Sichuan University	2020年审（163）号
1	Medical Research Ethics Committee of the First Affiliated Hospital of Nanchang University	（2020）医研伦审第（43）号
1	Medical Ethics Committee of Ganzhou Fifth People's Hospital	医伦审（2020）1号
1	Medical ethics committee of The First People’s Hospital of Jiangxia District	(2020)伦审批第013号

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1	Ethic Committee of Yueyang Hospital of Integrated Traditional Chinese and Western Medicine, Shanghai University of Traditional Chinese Medicine, Shanghai, China	2020-003
1	Medical Ethics Commitee, Zhongnan Hospital of Wuhan University	科伦【2020040】、科伦【2020040-1】
1	IRB of Shuguang Hospital affiliated with Shanghai University of TCM	2020-796-03-01
1	Shanghai Changzheng Hospital	2020SL002
1	Biomedical Research Ethics Committee，Ganzi Hospital of West China Hospital, Sichuan University	GZZYY-2020-02-10
1	Ethics Committee of Wuxi Fifth People's Hospital	2020-003-1
1	The Ethics Committee of First Affiliated Hospital of Harbin Medical University	IRB-AF/SC-04/01.0
1	Ethics Review Committee of China Academy of Chinese Medical Sciences	CACMS-IRB2020-002-1
1	The Ethical Committee of Tongji Hospital of Tongji Medical College; Huazhong Science and Technology University	TJ-C20200108
1	Biomedical ethics committee of West China Hospital of Sichuan University	2020年 审（194）号

For peer review only

0	#BLANK	#BLANK
1	Ethics committee of the second affiliated hospital of xi 'an medical university	X2Y202001
#NOTREQ	#NOTREQ	#NOTREQ
1	Ethics commitee of the first people's hospital of Jiangxia district, Wuhan	2020-0003
1	Medical Ethics Committee of Jingzhou First People's Hospital	Y20200211
1	Medical Ethics Committee of Wuhan Infectious Disease Hospital	KY-2020-26.02
1	Ethics Committee of Heilongjiang Social Rehabilitation Hospital	[2020]院伦审临医字第（001）号
1	Ethics review form for branch medical research and clinical technology application, ethics committee of First Affiliated Hospital of Fujian Medical University	闽医大附一伦理炎【2020】038
1	Clinical Trial Center of Zhongnan Hospital Affiliated to Wuhan University	20200228
1	Ethics Committee of the First Affiliated Hospital of Fujian Medical University	闽医大附一伦理医研[2020]039号

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1	EC of The Fourth People's Hospital of Wuhan	SY2020-027
1	Biomedical ethics committe of Dalian Medical University	2020004
1	IRB of Shuguang Hospital affiliated to Shanghai University of T.C.M.	20207970401
1	Ethic Committee of Maoming People's Hospital	2020013
1	Ethics Committee Longhua Hospital, Shanghai University of Traditional Chinese Medicine	2020LCSY008
1	Ethics Committee of Jiangsu Province Hospital on Integration of Chinese and Western Medicine	2020LWKY011
1	Ethics Committee of Jiangsu Province Hospital on Integration of Chinese and Western Medicine	2020LWKY009
1	Tongji Hospital, Tongji Medical College, Huazhong University of Science and Technology Institutional Review Board Approval	IRB ID:TJ-C20200109
1	Medical Ethics Committee， Zhongnan Hospital of Wuhan University	2020052
1	Ethics committee of the first affiliated hospital of guangzhou university of Chinese medicine	NO.ZYYECK【2020】017

For peer review only

1	Medical research ethics committee of Shiyan Taihe hospital	202001
#NOTREQ	#NOTREQ	#NOTREQ
1	Ethical Committee of The First Hospital of Harbin Medical University	IRB-AF/SC-04/01.0
1	Ethics Committee of the Second Affiliated Hospital of Wenzhou Medical University	LCKY2020-11
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1	Ethics Committee of Wuhan Red Cross Hospital	2020009
1	Medical ethics committee of Tongji Hospital Affiliated to Tongji Medical College of Huazhong University of science and technology	TJ-IRB20200318
1	Ethics Committee of Hubei Hospital of traditional Chinese Medicine	HBZY2020-A04-01
0	#BLANK	#BLANK
1	The Ethic Committee of Shanghai General Hospital	院伦审[2019]15

For peer review only

1	Ethics Committee of Scientific Research and Clinical Trials of the First Affiliated Hospital of Zhengzhou University	2020-KY-060
#NOTREQ	#NOTREQ	#NOTREQ
1	Medical ethics committee of Liaocheng People's Hospital	2020004
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1	Ethics Committee of Union Hospital affiliated to Tongji Medical College of Huazhong University of Science and Technology	[2020]伦审字（0083）号
1	Ethics Committee of research study of the Third People's Hospital of Shenzhen	深圳三院伦审科研字[2020-008-02]号
1	The Ethics Review Committee of Institute of Blood Transfusion, Chinese Academy of Medical Sciences	202003
1	Medical Ethics Committee of Xiangya Hospital Central South University	202003025
1	Medical ethics committee of the first people's Hospital of Jiangxia District	2020伦审批第14号
1	Medical Ethics Committee, Zhongnan Hospital of Wuhan University	2020019

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1	Medical Ethics Committee of Hubei Provincial Hospital of TCM	HBZY2020-C01-01
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1	Professional ethics committee for clinical research of The First Affiliated Hospital of Wenzhou Medical University	KY2020-003
1	Biomedical research ethics committee of Shanghai Changzheng Hospital	2020SL005
1	Ethic Committee of Maoming People's Hospital	2020012
#NOTREQ	#NOTREQ	#NOTREQ
1	Ethics Committee of Scientific Research Project Review of the First Affiliated Hospital of Guangzhou Medical University	医研伦审2020第59号
1	Human Research Ethics Committee of HwaMei Hospital, University of Chinese Academy of Sciences	PJ-NBEY-KY-2020-022-01
1	Human Research Ethics Committee of HwaMei Hospital, University of Chinese Academy of Sciences	PJ-NBEY-KY-2020-021-01
1	Ethics Committee of Foshan First People's Hospital	伦审研[2020]第3号

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1	IEC of Affiliated Hospital of Shaanxi University of Traditional Chinese Medicine	SZFYIEC-PJ-2020年第3号
1	Ethics Committee of Clinical Research, Renmin Hospital of Wuhan University	WDRY2020-K109
1	Medical Research Ethics Committee of the First Affiliated Hospital of Nanchang University	(2020)医研伦审第(47)号
1	Clinical Trial Ethics Committee of the First Affiliated Hospital of Guangzhou Medical University	EC-2020-016(YW)-02
1	Medical research ethics committee of the Second Affiliated Hospital of Nanchang University	(2020)医研伦审第（01）号
#NOTREQ	#NOTREQ	#NOTREQ
1	Ethical committee of Guangdong provincial hospital of Chinese medicine	AF2020-057-01
1	Medical Ethics Committee of Xiyuan Hospital, Chinese Academy of Traditional Chinese Medicine	2020XLA022-1
1	Ethic commitee of Ningbo Kangning Hospital	NBKNYY-2929-LC-08
1	Ethics Committee of the Third Xiangya Hospital of Central South University	快20040

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#NOTREQ	#NOTREQ	#NOTREQ
1	Ethics committee of Beijing You'an Hospital, Capital Medical University	京佑科伦字【2020】006号
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1	Medical ethics committee of the Third Affiliated Hospital of Sun Yat-sen University	[2020]02-037
1	Institutional Review Board of The First Teaching Hospital of Tianjin University of Traditional Chinese Medicine	TYLL2020[K]字009
#NOTREQ	#NOTREQ	#NOTREQ
1	Tongji Hospital, Tongji Medical College, Huazhong University of Science and Technology Institutional Review Board Approval	TJ-IRB20200389

For peer review only

1	Medical ethics committee of The Fifth Affiliated Hospital of Sun Yat-Sen University	中大五院[2020】伦字第K16-1号
#NOTREQ	#NOTREQ	#NOTREQ
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1	Ethics Committee of Beijing University of Chinese Medicine	2020BZHYLL0111
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1	Medical ethics committee of huzhou central hospital	20200214-01
1	Medical Ethics Commission of Henan Provincial Center for Disease Control and Prevention	2020-YM-002-02
1	Ethics committee of the First Affiliated Hospital with Nanjing Medical University	2020-SR-171
1	Guangzhou Eighth People's Hospital Ethic Committee	科 202010143

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#NOTREQ	#NOTREQ	#NOTREQ
1	Ethics Review Committee of China Academy of Chinese Medical Sciences	CACMS-IRB2020-004
1	Ethics Committee of Beijing University of Chinese Medicine	2020BZHYLL0113
1	Wuhan Huoshenshan Hospital Medical Ethics Committee	HSSLL018
1	Shanghai Public Health Clinical Center Ethics Committee	YJ-2020-S073-01
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1	Medical Ethics Committee of Hubei Provincial Hospital of TCM	HBZY2020-C21-01
1	Medical ethics committee of Wuhan 1st Hospital	武卫一院伦审【2020】9号
1	Ethics Committee of Hubei Provincial Hospital of TCM	HBZY2020-C27-01
1	Medical Ethics Committee of Mudanjiang Kang'an Hospital	医伦审202004号

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#NOTREQ	#NOTREQ	#NOTREQ
1	Scientific Research Ethics Committee of Beijing University of Chinese Medicine Third Affiliated Hospital	ECBJZYYKJFZZJSL-ZYDSY2020-01
1	Biomedical ethics committee, west China hospital, sichuan university	2020(206)
1	Ethics Committee of Yueyang Hospital of Integrated Traditional Chinese and Western Medicine, Shanghai University of Traditional Chinese Medicine	2020-002-01
1	Xiaogan Chinese medicine hospital ethics committee	HBXGZYY-2020-01
1	The Ethics Committee of Union Hospital affiliated to Tongji Medical College of Huazhong University of Science and Technology	N/A
1	Medical Ethics Commission of Henan Provincial Center for Disease Control and Prevention	2020-YM-001-02
1	Ethics Committee of Shanghai Public Health Clinical Center	2020-S063-01
1	The Ethical Committee of the Research Project Review of the First Affiliated Hospital of Guangzhou Medical University	2020-87
1	Medical Ethics Committee of Hubei Provincial Hospital of TCM	HBZY2020-C21-01

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1	Ethics Committee of the First Affiliated Hospital of Guangdong Pharmaceutical University	医伦审【2020】第（62）号
1	Ethics committee of Beijing Ditan Hospital affiliated to Capital Medical University	京地伦药字（2020）第（014）-02号
#NOTREQ	#NOTREQ	#NOTREQ
#NOTREQ	#NOTREQ	#NOTREQ
1	Medical Ethics Committee of Hongqi Hospital Affiliated to Mudangjiang Medical University	202014
1	Ethics Committee of Wuhan Third Hospital	武三医院KY2020-050
1	the Institutional Ethical Review Board of the First Hospital of China Medical University	2020-14-2
1	Ethics committee office of The First Affiliated Hospital of Wenzhou Medical University	（2020）第（002）
#NOTREQ	#NOTREQ	#NOTREQ
1	The Ethics Committee of the First Affiliated Hospital of Nanjing Medical University	2020-SR-149

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1	The ethic committee of Guangzhou Women and Children's Medical Center	202028500
1	The Medical Ethics Committee of Xiangyang General Hospital	XYSZXYY-LL-PJ-2020-001
1	Ethics Committee of Yueyang Hospital of Integrated Traditional Chinese and Western Medicine, Shanghai University of Traditional Chinese Medicine	2020-002-01
1	Shanghai Public Health Clinical Center Ethics Committee	[2020]YJ-2020-S065-02
#NOTREQ	#NOTREQ	#NOTREQ
1	Medical Ethics Committee of Children's Hospital Affiliated to Zhejiang University School of Medicine	2020-IEC-004
#NOTREQ	#NOTREQ	#NOTREQ
1	Ethics Committee of the First Affiliated Hospital of Bengbu Medical College	BYYFY-2020KY10
1	Shulan (Hangzhou) Hospital Ethics Committee for Clinical Trials	2020伦审第（18）号、GXIRB2020-0046
#NOTREQ	#NOTREQ	#NOTREQ

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#NOTREQ	#NOTREQ	#NOTREQ
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1	Ethics Committee of Hubei Hospital of Traditional Chinese Medicine	HBZY2020-C26-01
1	Shanghai Public Health Clinical Center Ethics Committee	YJ-2020-S124-02
#NOTREQ	#NOTREQ	#NOTREQ
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1	The Ethical Review Committee of Institute of Pathogen Biology, Chinese Academy of Medical Sciences	IPB-2020-22
1	Ethics Committee of Jiangsu Provincial Center for Disease Prevention and Control	JSJK2020-A039-01
1	Shanghai Ethics Committee for Clinical Research	SECCR/2020-20-03
#NOTREQ	#NOTREQ	#NOTREQ
1	IRB of The Third Affiliated Hospital of Beijing University of Traditional Chinese Medicine	BZYSY-2020KYKTPJ-07
1	IRB of The Third Affiliated Hospital of Beijing University of Traditional Chinese Medicine	BZYSY-2020KYKTPJ-06
#NOTREQ	#NOTREQ	#NOTREQ
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1	Research Ethics Committee of the Third Affiliated Hospital of Beijing University of Chinese Medicine	BZYSY-2020KYKTPJ-06
1	Ethic Committee of Jiangsu Provincial Center for Disease Control and Prevention	JSJK2020-A057-02

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#NOTREQ	#NOTREQ	#NOTREQ
1	Shulan (Hangzhou) Hospital Ethics Committee for Clinical Trials	2020伦审第（31）号
1	Ethics Committee of Xiangzhou District People's Hospital	202014
1	Ethic Committee of Jiangsu Provincial Center for Disease Control and Prevention	JSJK2020-A058-02
#NOTREQ	#NOTREQ	#NOTREQ
1	Ethics Committee of Jiangsu Provincial Center for Disease Prevention and Control	JSJK2020-A061-01
#NOTREQ	#NOTREQ	#NOTREQ
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For peer review only

1	Ethics Committee of Jiangsu Provincial Center for Disease Control and Prevention	JSJK2020-A063-02
1	Ethics Committee of Hu'nan Provincial Center for Disease Control and Prevention	湘疾控IRB2020040
#NOTREQ	#NOTREQ	#NOTREQ
1	Shanghai Public Health Clinical Center Ethics Committee	2020-S206-01
#NOTREQ	#NOTREQ	#NOTREQ
1	Ethics Committee of Shanxi Provincial Center for Disease Control and Prevention	SXCDCIRBPJ2020056001
1	Ethics Committee of Shanxi Provincial Center for Disease Control and Prevention	SXCDCIRBPJ2020056001
1	Ethics Committee of Shanxi Center for Disease Control and Prevention	SXCDCIRBPJ2020056001
1	Guangxi IRB	GXIRB2020-0066-1
1	Ethics Committee of Hebei Province Chest Hospital	2021002

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1	Ethics Committee of Hebei Province Chest Hospital	2021001
1	Ethics Committee of Hebei Province Chest Hospital	2021003
1	Shulan (Hangzhou) Hospital Ethics Committee for Clinical Trials	2021伦审第（1）号
1	#NOTREQ	#NOTREQ
1	Medical Ethics Committee of Tianjin University of TCM	TJUTCM-EC20210001
1	Research Ethics Committee of the Second Hospital of Hebei Medical University	2021-R013-01
1	Ethics Committee of Hubei Hospital of Traditional Chinese Medicine	HBZY2020-C27-01
#NOTREQ	#NOTREQ	#NOTREQ
1	Ethics Committee of Shanghai East Hospital	2020研审第096号
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For peer review only

#NOTREQ	#NOTREQ	#NOTREQ
1	Medical Ethics Committee of Chengdu Public Health Clinical Medical Center	PJ-K2021-01-01
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1	The Ethics Committee for Clinical Trials of Vaccines, Guangdong Provincial Center for Disease Control and Prevention	2021V002-F
1	The Ethics Committee for Clinical Trials of Vaccines, Guangdong Provincial Center for Disease Control and Prevention	2021V001-F
1	Ethics Committee of the First Affiliated Hospital, Sun Yat-Sen University	[2021]254
#NOTREQ	#NOTREQ	#NOTREQ
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1	Shulan (Hangzhou) Hospital Clinical Trial Ethics Committee	2021伦审第（3）号

For peer review only

#NOTREQ	#NOTREQ	#NOTREQ
1	SHANGHAI MUNICIPALCENTER FOR DISEASE CONTROL AND PREVENTION ETHICAL REVIEW COMMITTEE FOR CLINICAL TRIAL	2021-7
1	Ethics Committee of Hubei Provincial Center for Disease Control and Prevention (Hubei Academy of Preventive Medicine)	2021-008-02
1	Medical Ethics Committee of Tianjin University of Traditional Chinese Medicine	TJUTCM-EC20210001
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1	Ethics Committee of Jiangsu Provincial Center for Disease Prevention and Control	JSJK2021-A017-01
1	Ethics Committee of Shanghai East Hospital	2021研审第055号
1	Ethics Committee of Guangdong Provincial Hospital of Traditional Chinese Medicine	BF2021-113-01

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For peer review only

#NOTREQ	#NOTREQ	#NOTREQ
1	Ethics Committee of Hu'nan Center for Disease Control and Prevention	湘疾控 IRB-PJ2021013
1	Ethics Committee of Affiliated Hospital of Nanjing University of Chinese Medicine	2020NL-005-03
1	Ethical Committee of Affiliated Hospital of Nanjing University of Chinese Medicine (Jiangsu Province Hospital of Chinese Medicine)	2021NL-123-02
1	Institutional Ethics Committee of Guangdong Provincial Hospital of Chinese Medicine	ZE2021-049-01
1	Ethics Review Committee of China Academy of Chinese Medical Sciences	CACMS-IRB2021-003
1	Yunnan CDC IRB	批件2021-4号
1	Ethics Review Committee of Shanxi Provincial Center for Disease Control and Prevention	SXCDCIRBYJ2021062002
1	Ethics Committee of the First Affiliated Hospital of Chongqing Medical University	20214601
1	Clinical Trial Ethics Committee of Shulan (Hangzhou) Hospital	2021伦审第（20）号

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1	Guangxi IRB	GXIRB2021-0026
1	Ethics Committee of Hubei Provincial Hospital of Integrated Chinese and Western Medicine	2020伦审第（研019）号
#NOTREQ	#NOTREQ	#NOTREQ
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1	Huangshi Hospital of TCM Ethic Committee	HSZYPJ-2021-001-01
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1	Ethics Committee of Hubei Provincial Hospital of Integrated Chinese and Western Medicine	2020伦审第（研020）号
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For peer review only

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1	Medical Research Ethics Committee of Taihe Hospital, Shiyan City	科研会审（202015）号
1	Clinical Research Ethics Committee of Renmin Hospital of Wuhan University	WDRY2021-K122
1	Ethics Committee of Hunan Center for Disease Control and Prevention	湘疾控 IRB-YJ2021028
1	Ethics Committee of Beijing Ditan Hospital, Capital Medical University	2021-033-02
1	The Ethics Committee of Scientific Research and Clinical Trial, The First Affiliated Hospital of Zhengzhou University	L2021-Y266-002
1	Medical Ethics Committee of Shanghai Public Health Clinical Center	YJ-2020-S123-02
#NOTREQ	#NOTREQ	#NOTREQ

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1	Guangxi Institutional Review Board	GXIRB2021-0042-1
#NOTREQ	#NOTREQ	#NOTREQ
1	Ethics Committee of Sichuan Provincial Center for Disease Control and Prevention	SC-0820212401
1	Medical Ethics Committee of Shanghai Public Health Clinical Center	2021-E020-02号
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1	Ethics Committee of Vaccine Clinical Trial, Yunnan Center for Disease Control and Prevention	批件2021-3号

For peer review only

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1	Ethics Committee of Hubei Provincial Hospital of TCM	HBZY2020-C27-01
1	Vaccine Clinical Trial Ethics Committee of Sichuan Center for Disease Control and Prevention	SC-0820212101
1	Ethics Committee of Beijing Ditan Hospital, Capital Medical University	京地伦科字〔2021〕第（024）-02号
1	YNIRB	批件2021-15号
1	Ethics Committee, School of Nursing, Yangzhou University	YZUHL2021017
1	Ethics Committee of Jiangsu Provincial Center for Disease Control and Prevention	JSJK2021-A034-02
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1	Medical Ethics Committee of Changde First People's Hospital	2021-077-01

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Ethic Committee Approval Date	Multicenter Flag (Yes = 1: if Number of Centers Declared by Investigator >1; No = 0)	Number of Centers Declared by Investigator
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2020/2/29	1	12
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For peer review only

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For peer review only

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For peer review only

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For peer review only

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For peer review only

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For peer review only

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For peer review only

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For peer review only

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For peer review only

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For peer review only

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For peer review only

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For peer review only

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For peer review only

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For peer review only

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Center name
Wuhan Jinyintan Hospital (Wuhan Infectious Diseases Hospital)
Medical ICU,Peking Union Medical College Hospital
The Fifth Medical Center, General Hospital of PLA
Chongqing Public Health Medical Center
Chongqing Public Health Medical Center
Shanghai Public Health Clinical Center
Hubei Provincial Hospital of Integrated Chinese and Western Medicine
Medical Center for Digestive Diseases, The Second Affiliated Hospital of Nanjing Medical University
Guangzhou Eighth People's Hospital

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Jin Yin-tan hospital

Bin Cao

Affiliated Zhongshan Hospital of Dalian University
Zhongnan Hospital of Wuhan University

Ninth Hospital of Wuhan
Shijiazhuang Fifth Hospital
Beijing YouAn Hospital
Henan provincial hospital for infectious diseases
The Second Affiliated Hospital of Chongqing Medical University
The No. 9 Hospital of Wuhan
The Central Hospital of EZhou
Fuyang Second People's Hospital
People's Hospital of Guizhou
The Infectious Disease hospital of Harbin

Wuhan 1st Hospital

Hubei integrated hospital of traditional Chinese and Western Medicine

Hubei integrated traditional Chinese and Western Medicine Hospital
WUHAN HOSPITAL OF TCM

Xinhua affiliated hospital, Hubei University of Chinese Medicine
Hubei Provincial Hospital of Integrated Chinese and Western Medicine

Xinhua affiliated hospital, Hubei University of Chinese Medicine
Hubei Provincial Hospital of Integrated Chinese and Western Medicine

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Xinhua affiliated hospital, Hubei University of Chinese Medicine
Hubei Provincial Hospital of Integrated Chinese and Western Medicine

Xinhua affiliated hospital, Hubei University of Chinese Medicine
Hubei Provincial Hospital of Integrated Chinese and Western Medicine

Xinhua affiliated hospital, Hubei University of Chinese Medicine
Hubei Provincial Hospital of Integrated Chinese and Western Medicine

Department and Institute of Infectious Disease

Department and Institute of Infectious Disease

Department and Institute of Infectious Disease

The First Hospital of Changsha
The Second Xiangya Hospital of Cental South University

Hangzhou Xixi Hospital Affiliated to Zhejiang Chinese Medical University

Xixi Hospital of Hangzhou

Tongji Hospital, Tongji Medical College, Huazhong University of Science and Technology

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Zhongnan Hospital of Wuhan University

The First Affiliated Hospital, Zhejiang University School of Medicine

The First Affiliated Hospital, Zhejiang University School of Medicine

Hospital of Chengdu University of Traditional Chinese Medicine

Renmin Hospital of Wuhan University

Xiangyang 1st People's Hospital

Department and Institute of Infectious Disease

Zhongnan Hospital of Wuhan University

Xiangyang First People's Hospital

The First Affiliated Hospital of Medical College of Zhejiang University

For peer review only

Tongji Hospital, Tongji Medical College, Huazhong University of Science and Technology

Hubei Integrated Traditional Chinese and Western Medicine Hospital
Huanggang Central Hospital (Dabieshan Medical Center)
People's Hospital of Guangshan County
Hanchuan People's Hospital
The First People's Hospital of Jiangxia District

Hubei Provincial Hospital of TCM

Hubei Provincial Hospital of TCM

The First Affiliated Hospital, Zhejiang University School of Medicine
The First Hospital of Jiaxing
Huzhou Central Hospital
The First People's Hospital of Xiaoshan District, Hangzhou
Wenzhou Central Hospital
Infectious Disease Hospital of Heilongjiang Province
Tongji Hospital
Harbin infectious diseases hospital
Anhui Province HOspital
The second hospital of Nanjing

Shanghai Public Health Clinical Center

The First Affiliated Hospital, College of Medicine, Zhejiang University

West China Hospital, Sichuan University
Chengdu Public Health Clinical Medical Center
Wuhan Red Cross Hospital
The Affiliated Hospital of Southwest Medical University
Guangqu District, the Maternal and Child Health Hospital of Hubei Province

Wuhan Pulmonary Hosptial

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The Fourth People's Hospital of Wuhan

TongJi Medical College of HUST

The Fifth Affiliated Hospital Sun Yat-Sen University

Tongji Hospital,Tongji Medical College, Huazhong University of Science and Technology
Institutional

The First Affiliated Hospital of Anhui University of Traditional Chinese Medicine
The First Affiliated Hospital Anhui Medicine University
Sixian Hospital of Traditional Medicine
Anhui Provincial Hospital

Tongji Hospital

Renmin Hospital of Wuhan University

Union Hospital, Tongji Medical College, Huazhong university of Science and Technology
Red Cross Hospital in Wuhan of Hubei Province
Wuhan Blood Center
Tongji Hospital, Tongji Medical College, Huazhong university of Science and Technology
Wuhan Asia Heart Hospital

The Second Affiliated Hospital of ChongQing Medical University
Three Gorges Central Hospital

Chongqing Public Health Medical Center
Three Gorge Central Hospital

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Chongqing Public Health Medical Center
Yongchuan Hospital Affiliated to Chongqing Medical University
Chongqing Shanxia Central Hospital
People's Hospital of Qianjiang District

Chongqing Public Health Medical Center
Yongchuan Hospital Affiliated to Chongqing Medical University
Chongqing Shanxia Central Hospital
People's Hospital of Qianjiang District

Wuhan Keting Square Cabin Hospital

The First Affiliated Hospital of University of science and technology of China (Anhui Provincial Hospital)

Zhongnan Hospital of Wuhan University

Enze Hospital of Taizhou Enze Medical Center (Group)

The First Affiliated of Wenzhou Medical University
The Second Affiliated of Wenzhou Medical University

Huangshi Hospital of traditional Chinese Medicine
Tongji Hospital of Huazhong University of science and technology
LaoHeKou Tradiational Chinese Medicine Hospital
Huanggang Central Hospital
Xishui County People's Hospital
Huangmei County People's Hospital
Wuhan Puren Hospital
Qichun County People's Hospital

Wuhan Puren Hospital
Huanggang Central Hospital
Xishui County People's Hospital
Huangmei County People's Hospital
Xiaogan Central Hospital

For peer review only

The First Clinical Hospital of Beijing University of Chinese Medicine (Dongzhimen Hospital)

The First Affiliated Hospital of He'nan University of Chinese Medicine
Zhengzhou Sixth People's Hospital
Nanyang Central Hospital
The First Affiliated Hospital of Nanyang Medical College
Zhumadian Central Hospital

Shanghai Pulmonary Hospital

The First Affiliated Hospital of Xinxiang Medical University

Renmin Hospital of Wuhan University

Wuhan Jinyintan Hospital (Wuhan Infectious Diseases Hospital)

Shanghai Public Health Clinical Center

Nanjing Second Hospital

Jingzhou Centeral Hospital

Union Hospital, Tongji Medical College, Huazhong University of Science and Technology

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The Second Affiliated Hospital of Wenzhou Medical University

Jingzhou Centeral Hospital

The First Affiliated Hospital of Zhengzhou University

Wuhan Jinyintan Hospital
Renmin's Hospital of Wuhan University
Tongji Hospital Affiliated to Huazhong University of science and technology

People's Hospital of Guangshan County

The First Affiliated Hospital of Medical College of Zhejiang University

Beijing You'an Hospital, Capital Medical University
Huangshi Central Hospital of Hubei
Fuyang Second People's Hospital, Anhui
The Second Affiliated Hospital of Harbin Medical University
Heilongjiang Provincial Hospital
Suizhou Central Hospital
Xiaogan Central Hospital
Wuhan 7th Hospital
Ezhou Central Hospital
Jingzhou First People's Hospital
Huangshi Hospital of Traditional Chinese Medicine
Tongji Hospital of Huazhong University of science and technology
LaoHeKou Tradiational Chinese Medicine Hospital
Hubei province hospital of integrated Chinese & Western Medicine
Yichang first people's Hospital
Beijing YouAn Hospital
Renmin Hospital of Wuhan University
Tianyou Hospital Affiliated to Wuhan University of science and technology

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Shuguang Hospital

Tongji Hospital of Tongji Medical College, Huazhong University of Science and Technology
The First Affiliated Hospital of Nanchang University
Beijing Haidian Hospital (Haidian Section of Peking University Third Hospital)

Tongji Hospital of Tongji Medical College, Huazhong University of Science and Technology
The First Affiliated Hospital of Nanchang University
Beijing Haidian Hospital (Haidian Section of Peking University Third Hospital)

HwaMei Hospital, University of Chinese Academy of Sciences

Wuhan Pulmonary Hospital
Wuhan Third People's Hospital
Hefei Infectious Disease Hospital
Jiangsu Province Infectious Disease Hospital
The First Affiliated Hospital of Xi'an Jiaotong University

Wuhan Pulmonary Hospital
Wuhan Third People's Hospital
Hefei Infectious Disease Hospital
Jiangsu Province Infectious Disease Hospital
The First Affiliated Hospital of Xi'an Jiaotong University

Union Hospital affiliated to Tongji Medical College of Huazhong University of Science and Technology
Hubei Hospital of traditional Chinese Medicine
Suizhou Central Hospital
The third people's Hospital of Hubei Province

Hospital of Chengdu University of Traditional Chinese Medicine

Tongji Hospital, Tongji Medical College, Huazhong University of Science and Technology

Shandong Province Chest Hospital
Jinan Infectious Disease Hospital
Jining Infectious Disease Hospital
Qingdao Chest Hospita
Yantai Qishan hospital

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Yueyang Hospital of Integrated Traditional Chinese and Western Medicine, Shanghai University of Traditional Chinese Medicine
Huangshi Hospital of Traditional Chinese Medicine

Zhongnan Hospital of Wuhan University

Central theater General Hospital Wuchang district
Central theater General Hospital Hankou district
Huoshenshan hospital

beijing youan houspital
Wuhan PuRen Hospital
Shanghai Guanghua integrated traditional Chinese and Western Medicine Hospital

The First Affiliated Hospital of Nanchang Uninversity
Foshan 4th People's Hospital
Ezhou City Central Hospital

Guanggu District, Tongji Hospital, Tongji Medical College, Huazhong University of Science and Technology

The First Affiliated Hospital of Guangzhou Medical University
Xianning Central Hospital
First People's Hospital of Yunnan Province
Wuhan Sixth Hospital

Huangshi city hospital of TCM

The first affiliated hospital of Harbin medical university
The second affiliated hospital of Harbin medical university
The fourth affiliated hospital of Harbin medical university
Heilongjiang Provincial hospital

The First Affiliated Hospital of University of science and technology of China (Anhui Provincial Hospital)

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Hospital of Chengdu University of Traditional Chinese Medicine

Union Hospital, Tongji Medical College, Huazhong University of Science and Technology

The First Affiliated Hospital of Guangzhou Medical University
Guangzhou Eighth People's Hospital
Union Hospital, Tongji Medical College, Huazhong University of Science and Technology
Wuhan Hankou Hospital

Wuhan Jinyintan Hospital (Wuhan Infectious Diseases Hospital)

The fourth people's hospital of nanning, guangxi

Jingzhou Hospital of Traditional Chinese Medicine
Wuhan Leishenshan (Thunder God Mountain) Hospital/The First Affiliated Hospital of Dalian Medical University
Xiangyang Hospital of Traditional Chinese Medicine

Beijing Chaoyang Hospital, Capital Medical University

Wuhan Fourth Hospital

First Affiliated Hospital of Anhui Medical University
Wuhan mobile cabin hospital
Binhu hospital of the first people's hospital of hefei
The second people's hospital of Hefei
Suzhou hospital affiliated to anhui medical university

Renmin Hospital of Wuhan University

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The Sixth Affiliated Hospital of Guangzhou Medical University (Qingyuan People's Hospital)
Wuhan Dongxihu Fangcang Hosptial

The First Affiliated Hospital of Guangzhou Medical University

The first affiliated hospital of wenzhou medical university
Wenzhou central hospital
The first people's hospital of Wenling
Shaoxing people's hospital
Lishui zhongyi yiyuan
The first Affiliated Hospital of AnHui Medical University
Anhui Provincial Hospital for Infection
The First Affiliated Hospital of Bengbu Medical College
Henan Provincial People's Hospital
The First Affiliated Hospital of Zhengzhou University

GCP Office of The First Affiliated Hospital of Guangzhou Medical University

Guanggu District, Tongji Hospital, Tongji Medical College, Huazhong University of Science and Technology

The First Affiliated Hospital of Guangzhou Medical University
Wuhan University People's Hospital
Union Hospital Affiliated to Tongji Medical College of Huazhong University of science and technology
Wuhan Huangpi District Hospital of traditional Chinese Medicine
Renmin Hospital of Wuhan University
Zhongnan Hospital of Wuhan University
The Third Hospital of Wuhan City

Cancer Center, Xiehe Hospital,Tongji Medical College, Huazhong University of Science and Technology

People's Hospital of Wuhan University
Central Hospital of Chongqing Three Gorges
The First Affiliated Hospital of Chongqing Medical University
The Fourth People's Hospital of Taiyuan

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The sixth medical center of PLA general hospital
The 5th medical center of PLA general hospital

Shanghai Changzheng Hospital

Tianjin Haihe Hospital
Zhongnan Hospital
Enshi Central Hospital

the first affiliated hospital of Nanchang university

Shanghai Public Health Clinical Center

Wuhan Longyang Hospital

Tongji Hospital, Tongji Medical College, Huazhong University of Science and Technology
The Second Affiliated Hospital of Xi'an Jiaotong University

Hainan Medical University

The 5th Medical Center Chinese PLA General Hospital

Renmin Hospital of Wuhan University

For peer review only

Nanhua Hospital Affiliated to Nanhua University

The First Affiliated Hospital of Nanchang University

Jingzhou first people's Hospital

Hubei Provincial Hospital of Integrated Traditional Chinese and Western Medicine

General Hospital of Central Theater Command
Maternal and Child Hospital of Hubei Province
Wuhan Huoshenshan Hospital

Puren Hospital Affiliated to Wuhan University of Science and Technology

West China Hospital of Sichuan University

The First Affiliated Hospital of Nanchang University, Ten layers pulse Research Institute of traditional Chinese medicine, Nanchang University

Fifth People's Hospital of Ganzhou

The First People’s Hospital of Jiangxia District
Union Hospital, Tongji Medical College, Huazhong University of Science and Technology

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Wuhan Leishenshan Hospital

Zhongnan Hospital of Wuhan University
Leishenshan Hospital of Wuhan
The Third People's Hospital of Hubei Province

Leishenshan Hospital

Shanghai Changzheng Hospital

Ganzi Hospital of West China Hospital, Sichuan University

Wuxi Fifth People's Hospital

the First Affiliated Hospital of Harbin Medical University

Wuhan Jinyintan Hospital

Jingzhou Central Hospital

West China Hospital, Sichuan University

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Union Hospital, Tongji Medical College, Huazhong University of Science & Technology

The second affiliated hospital of xi 'an medical university
Shaanxi provincial hospital for infectious diseases

Ruijin Hospital Affiliated to Shanghai Jiao Tong University School of Medicine

First people's hospital of Jiangxi district, Wuhan

Jingzhou First People's Hospital

Wuhan Jinyintan Hospital (Wuhan Infectious Diseases Hospital)

Harbin infectious diseases hospital

The First Affiliated Hospital of Fujian Medical University

Wuhan Raytheon mountain hospital (Zhongnan hospital, wuhan university)

The First Affiliated Hospital of Fujian Medical University

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The Fourth People's Hospital of Wuhan

Wuhan Raytheon mountain hospital (Zhongnan hospital, wuhan university)

Leishenshan hospital

Maoming People's Hospital
Guangdong Provincial People's Hospital
Wuhan City Hankou Hospital
Jingzhou Central Hospital
Huizhou Central Hospital

Leishenshan Hospital
mobile cabin hospital

Affiliated Hospital of traditional Chinese and Western Medicine, Nanjing University of Chinese Medicine
Wuhan Jiangxia District First People's Hospital
Wuhan Jiangxia District Traditional Chinese Medicine Hospital
Affiliated Hospital of traditional Chinese and Western Medicine Nanjing University of Chinese Medicine
Wuhan Jiangxia District First People's Hospital
Wuhan Jiangxia District Traditional Chinese Medicine Hospital

Tongji Hospital

Zhongnan Hospital of Wuhan University
Wuhan Central Hospital
Wuhan Seventh Hospital

Hubei provincial hospital of integrated traditional Chinese and western medicine
Wuhan Hospital of Traditional Chinese Medicine

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Hubei Shiyan Taihe hospital

Tetrandrine Tablets

The First Hospital of Harbin

Ethics Committee of the Second Affiliated Hospital of Wenzhou Medical University

Wuhan Red Cross Society Hospital
Hospital of Chengdu University of Traditional Chinese Medicine
Chengdu Fifth People's Hospital

Wuhan Red Cross Hospital

Tongji Hospital, Tongji Medical College, Huazhong University of Science and Technology

Hubei Hospital of Traditional Chinese Medicine
Xiaogan first people's Hospital
Xianning first people's Hospital
Xiehe Jiangnan hospital
The third people's Hospital of Hubei Province

Shandong Provincial Hospital

Wuhan Leishenshan Hospital

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The First Affiliated Hospital of Zhengzhou University

Guiqiang Wang
Peking University First Hospital
Ezhou Central Hospital
Huoshenshan Hospital of Wuhan
Jinyintan Hospital of Wuhan

Liaocheng people's hospital

Qilu Hospital of Shandong University

Union Hospital affiliated to Tongji Medical College of Huazhong University of Science and Technology

The Third People's Hospital of Shenzhen

Union Hospital, Tongji Medical College, Huazhong university of Science and Technology
Red Cross Hospital in Wuhan of Hubei Province
Wuhan Asia Heart Hospital
Wuhan Maternal and Child Health Hospital

Xiangya Hospital (West District of Wuhan Union Medical College Hospital)
Third Xiangya Hospital (Sino-French New Hospital District of Wuhan Tongji Hospital)

The first people's Hospital of Jiangxia District, Wuhan

Zhongnan Hospital of Wuhan University

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Hubei Provincial Hospital of TCM

Shenzhen Third People's Hospital (infection department 2)
Shenzhen People's Hospital (infection department)

The First Affiliated Hospital of Wenzhou Medical University
Wenzhou city center hospital medical group
The second hospital affiliated to wenzhou medical college

Shanghai Changzheng Hospital
Guanggu District, Hubei maternal and Child Health Hospital

Maoming People's Hosptial

Wuhan No.7 Hospital/Jiznong Energy Fengfeng Group Hospital
Wuhan No.7 Hospital/North China University of Science and Technology Affiliated Hospital
Zhongnan Hospital of Wuhan University/Tanshan People's Hospital
Wuhan Leishenshan (Thunder God Mountain) Hospital/The First Affiliated Hospital of Dalian Medical University

The First Affiliated Hospital of Guangzhou Medical University

HwaMei Hospital, University of Chinese Academy of Sciences

HwaMei Hospital, University of Chinese Academy of Sciences

Foshan First People's Hospital
Zhongshan Second People's Hospital
The Ninth People's Hospital of Dongguan
Jiangmen Central Hospital
Foshan Fourth People's Hospital

For peer review only

Affiliated Hospital of Shaanxi University of Traditional Chinese Medicine

Renmin Hospital of Wuhan University

The First Affiliated Hospital of Nanchang University
Xinyu People's Hospital
Jiujiang No.1 People's Hospital
The First Affiliated Hospital of Guangzhou Medical University
Zhengzhou Sixth People's Hospital
Public Health Clinical Center of Chengdu
The First Hospital of Qiqihar
Heze Municipal Hospital

The Second Affiliated Hospital of Nanchang University
Jiangxi Chest Hospital

Beijing Chaoyang hospital

Hospital of Leishenshan

Wuhan Jinyintan Hospital

Wuhan Tongji Hospital Optical Valley Branch

The Third Xiangya Hospital of Central South University
The Fourth People's Hospital of Yueyang

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Shanghai Public Health Clinical Center

Guangzhou Institute of Respiratory Health, The First Affiliated Hospital of Guangzhou Medical University

Beijing You'an Hospital, Capital Medical University
Peking University First Hospital
Beijing huimin hospital

First Affiliated Hospital of Guangzhou Medical University

Chongqing Public Health Medical Center

Fuzhou General Hospital

The Third Affiliated Hospital of Sun Yat-sen University

Wuhan Jiangxia Cabin Hospital

Renmin Hospital of Wuhan University (East Campus)

Tongji Hospital of Tongji Medical College Huazhong Science and Technology University

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The fifth affiliated hospital of sun yat-sen university

The Second People's Hospital of Fuyang
Ezhou Hospital of Traditional Chinese Medicine
Ezhou Central Hospital
Huoshenshan Hospital of Wuhan
Jinyintan Hospital of Wuhan
The Second People's Hospital of Fuyang
Ezhou Central Hospital
Huoshenshan Hospital of Wuhan
Jinyintan Hospital of Wuhan
Tongji Hospital of Huazhong University of Science and Technology

Leishenshan hospital

Huangzhou District People's Hospital of Huanggang City

Hubei Provincial Center for Disease Control and Prevention

Huzhou central hospital

Wuzhi County Center for Disease Control and Prevention
People’s hospital, Wuzhi county

Jiangsu Province Hospital
Hubei Province Hospital Intergrated Chinese & Western Medicine
Huangshi Hospital of Chinese Traditional Medcine
Guangzhou Eighth People's Hospital
Maoming People's Hospital
Shenzhen Third Peoples Hospital
Yuebei Second People's Hospital
Dongguan Ninth People's Hospital

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Suining County Center for Disease Control and Prevention

China Academy of Chinese Medical Sciences?

Huangzhou District People's Hospital of Huanggang

Huoshenshan Hospital

Shanghai Public Health Clinical Center

Hubei Province Hospital

Hubei Provincial Hospital of TCM
Wuchang District Shuiguohu street community health service center
Wuchang District Xujiapeng street community health service center

Wuhan Hospital Of Traditional Chinese and Western Medicine
Wuhan No. 7 hospital
Xiaogan Traditional Chinese Medicine Hospital

Hubei Provincial Hospital of TCM
Wuhan Third Hospital
Xiaogan Chinese Medical Hospital

The First Affiliated Hospital of Harbin Medical University
Hongqi Hospital Affiliated to Mudanjiang Medical College

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Cai Yue

Beijing University of Chinese Medicine Third Affiliated Hospital
China-Japan Friendship Hospital
Berjing Dong zhi men Hospital
West China Hospital, Sichuan University
Changhai hospital
Wuhan Red Cross hospital
East hospital of people's hospital of Wuhan University
Wuhan Wuchang hospital
Yueyang Integrated Traditional Chinese and Western Medicine Hospital Affiliated to Shanghai University of Traditional Chinese Medicine
Huangshi City Hospital of Traditional Chinese Medicine

Xiaogan Traditional Chinese Medical Hospital
Hubei Provincial Hospital of TCM
Ezhou hospital of traditional Chinese medicine

West Campus of Union Hospital affiliated to Tongji Medical College of Huazhong University of Science and Technology

Liangyuan district centers for disease control and prevention

Wuhan Jinyintan hospital

The First Affiliated Hospital of Guangzhou Medical University
Xiangzhou District People's Hospital
Third Hospital of Baotou City
Wuhan Pulmonary Hospital
Xiangyang Central Hopital

Hubei Provincial Hospital of TCM

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The First Affiliated Hospital of Guangdong Pharmaceutical University
Guangzhou Eighth People's Hospital

Shanghai Public Health Clinical Center
Guangzhou 8th People's Hospital
TIANJIN HAIHE HOSPITAL
The First Affiliated Hospital of Harbin Medical University
Harbin Sixth Hospital

Renqiu City Center for Disease Control and Prevention

Nanjing Drum Tower Hospital

Hongqi Hospital Affiliated to Mudangjiang Medical University

Wuhan Third Hospital
Wuhan Integrated Traditional Chinese and Western Medicine Hospital (Wuhan First Hospital)
Ezhou Hospital of Traditional Chinese Medicine

First Hospital of China Medical University
China-Japan Friendship Hospital
Xiangya Second Hospital, Central South University

The First Affiliated Hospital of Wenzhou Medical University

West China Second University Hospital, Sichuan University
West China women's and children's Hospital

Wuhan Sports Center makeshift hospital
Jiangsu Province Hospital

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Guangzhou Women and Children's Medical Center
Guangzhou Eighth People's Hospital

Xiangyang Central Hospital
First People's Hospital of Xingyang

Yueyang Integrated Traditional Chinese and Western Medicine Hospital Affiliated to Shanghai University of Traditional Chinese Medicine
Huangshi City Hospital of Traditional Chinese Medicine

Shuguang Hospital Affiliated to Shanghai University of Traditional Chinese Medicine

Huashan Hospital affiliated to Fudan University

Children's Hospital Affiliated to Zhejiang University School of Medicine

The Second Affiliated Hospital of Chongqing Medical University

The First Affiliated Hospital of Bengbu Medical College

Shulan (Hangzhou) Hospital
Yongfu County Center for Disease Control and Prevention

Hunan Provincial Center for Disease Control and Prevention

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West China Second University Hospital, Sichuan University
West China women's and children's Hospital

Investigative Site

Investigative Site

Beijing SHIJITAN Hospital

Hubei Hospital of Traditional Chinese Medicine

Shanghai Public Health Clinical Center

Hunan Provincial Center for Disease Control and Prevention

Jiangsu Provincial Center for Disease Control and Prevention

Shanghai Public Health Clinical Center

Jiangsu Provincial Center for Diseases Control and Prevention

For peer review only

Wuhan Jinyintan Hospital

Dongtai Center for Disease Control and Prevention

Huashan Hospital Affiliated to Fudan University

Zanhuang county Center for Disease Control and Prevention

Hubei 672 Orthopaedic Hospital of Intergrated Traditional Chinese and Western Medicine
The Third Affiliated Hospital of Beijing University of traditional Chinese Medicine

Hubei 672 Orthopaedic Hospital of Integrated Traditional Chinese and Western Medicine
Beijing University of Chinese Medicine Third Affiliated Hospital

Taixing City center for Disease Control and Prevention

Beijing Ditan Hospital affiliated to Capital Medical University
Huashan Hospital affiliated to Fudan University

Hubei 672 Orthopaedics Hospital of Integrated Chinese and Western Medicine

Huaiyin District Center for Disease Control and Prevention, Huaian City

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Guangzhou Institute of Respiratory Disease
First Affiliated Hospital of Guangzhou Medical University

Shulan (Hangzhou) Hospital

Xiangyang Central Hospital
Xiangyang Hospital of Traditional Chinese Medicine

Huaiyin District Center for Disease Control and Prevention, Huaian City

Dafeng District Center for Disease Prevention and Control

Jiangsu Dongtai Center for Disease Control and Prevention

Shanghai Public Health Clinical Center

Jiangsu Provincial Center for Disease Control and Prevention

Jiangsu Provincial Center for Diseases Control and Prevention

Jiangsu Provincial Center for Diseases Control and Prevention

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Danyang City Center for Disease Control and Prevention

Xiangtan County Center for Disease Control and Prevention

Xi'an International Medical Center Hospital

Shanghai Public Health Clinical Center

Investigative Site

Taiyuan Center for Disease Control and Prevention
Shanxi Bethune Hospital
Taiyuan People's Hospital
Taiyuan Fourth People's Hospital

Outpatient Department of Shanxi Aviation Industry Group Co. LTD

Outpatient Department of Shanxi Aviation Industry Group Co. LTD

Yongfu CDC
Xiangfen CDC

The Second Hospital of Xingtai
Harbin Infectious Disease Hospital

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The Second Hospital of Xingtai
Harbin Infectious Disease Hospital

The Second Hospital of Xingtai
Harbin Infectious Disease Hospital

Shu Lan (Hang Zhou) Hospital

Jiangsu Provincial Center for Diseases Control and Prevention

Shijiazhuang Fifth Hospital
Shijiazhuang People's Hospital

The Second Hospital of Hebei Medical University

Hubei Provincial Hospital of Traditional Chinese Medicine

Beijing Ditan Hospital Capital Medical University
Guangzhou Eighth Hospital, Guangzhou Medical University
The First Affiliated Hospital of Guangzhou Medical University
The Third People's Hospital of Shenzhen
Subei People's Hospital of Jiangsu province

Shanghai Oriental Hospital

Kaihua county Center for Disease Control and Prevention

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Jiangsu Provincial Center for Diseases Control and Prevention

Public Health Clinical Center of Chengdu (Chengdu Infectious Disease Hospital)

Jiangsu Provincial Center for Diseases Control and Prevention

Gaozhou Municipal Center for Disease Control and Prevention

Gaozhou Municipal Center for Disease Control and Prevention

The First Affiliated Hospital, Sun Yat-Sen University

Jiangsu Provincal Center for Disease Control and Prevention

Ning ling Center for Disease Control and Prevention

Yanjin County Center for Disease Control and Prevention

shulan (Hangzhou) Hospital

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Zanhuang county Center for Disease Control and Prevention

Yuepu Community Health Service Center
Kangjian Community Health Service Center
Xinzhuang Community Health Service Center
Dongjing Town Community Health Service Center
Sijing Town Community Health Service Center

Xiangzhou District Center for Disease Control and Prevention
Liangyuan District Center for Disease Control and Prevention
Yangchun Center for Disease Control and Prevention

Shijiazhuang People's Hospital
The Fifth Hospital of Shijiazhuang City
Xingtai Second People's Hospital

General Hospital of Northern Theater Command

Funing Center for Disease Control and Prevention

Jiangsu Provincial Center for Disease Control and Prevention

Jiangsu Dongtai Center for Disease Control and Prevention

Shanghai East Hospital

the Eighth Affiliated Hospital of Guangzhou Medical University

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Shanxi Center for Disease Control and prevention
Sichuan Center for disease control

The Second Hospital of Anhui Medical University

Rushan City Center for Disease Control and Prevention

Hunan Provincial Center for Disease Control and Prevention

Xiangcheng Center for Disease Control and Prevention

Xiangcheng Center for Disease Control and Prevention

Huaiyin Center for Disease Control and Prevention
Binhai Center for Disease Control and Prevention
Xiangshui Center for Disease Control and Prevention

Hunan Provincial Center for Diseases Control and Prevention

Hunan Provincial Center for Diseases Control and Prevention

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Jiangsu Provincial Center for Diseases Control and Prevention

Xiangtan County Center for Disease Control and Prevention

Wuhan Jiangxia District Hospital of Traditional Chinese Medicine
Wuhan First Hospital
Huai 'an Traditional Chinese Medicine Hospital
Zhenjiang Infectious Disease Hospital
Yangzhou Hospital of Traditional Chinese Medicine

The Second Hospital of Nanjing
Yangzhou Third People's Hospital

Jiangsu Provincial Hospital of Chinese Medicine
The second hospital of Nanjing

Jiangsu Privince Hospital of Chinese Medicine(centralized isolation site under its management)
Yangzhou Hospital of Traditional Chinese Medicine (centralized isolation site under its management)

Center for Disease Control and Prevention of Xiangyun County

Yuncheng Yanhu District Center for Disease Control and Prevention

The First Affiliated Hospital of Chongqing Medical University

Shulan(Hangzhou)Hospital

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Yongfu County Center for Disease Control and Prevention
Liujiang District Center for Disease Control and Prevention
Yun County Center for Disease Control and Prevention

Hubei Provincial Hospital of Integrated Chinese and Western Medicine

Ruijin Hospital, Medical School of Shanghai Jiaotong University

Shanghai Public Health Clinical Center

Huangshi Hospital of TCM

Donghai County Center for Diseases Control and Prevention

Hubei Provincial Hospital of Integrated Chinese and Western Medicine

Jiangsu Provincial Center for Diseases Control and Prevention

Beijing Centers for Diseases Control and Prevention

Guangdong Center for Disease Control and Prevention
Henan Center for Disease Control and Prevention
Hubei Center for Disease Control and Prevention

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Shi Jiazhuang People's Hospital
The First Affliated Hospital of Harbin Medical University
Suihua first hospital

Xinqi City Center for Disease Control and Prevention

Hunan Provincial Center for Disease Control and Prevention

Taihe Hospital of Shiyan City

Wuhan Jinyintan Hospital

Xiangtan County Center for Disease Control and Prevention

Beijing Ditan Hospital, Capital Medical University

The First Affiliated Hospital of Zhengzhou University

Shanghai Public Health Clinical Center

Hunan Provincial Center for Disease Control and Prevention

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Liuzhou County Center for Disease Control and Prevention

Yangchun City Center for Disease Control and Prevention
Zhanjiang Center for Diseases Control and Prevention

Neijiang City Center for Disease Control and Prevention

Shanghai Public Health Clinical Center

Yaliang county Center for Disease Control and Prevention

Huai'an No.4 People's Hospital
Taizhou People's Hospital
Wuxi No.5 People's Hospital
Xuzhou Infectious Disease Hospital
Binhai County Hospital of Chinese medicine

Yanhe Tujia Autonomous County Center for Disease Control and Prevention

Beijing Youan Hospital Capital Medical University

The Second Affiliated Hospital of Guangzhou Medical University

Center for Disease Control and Prevention of Xiangyun County

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Huashan Hospital affiliated to Fudan University

Beijing Pinggu Hospital
Peking University Health Science Center Hospital

Beijing Pinggu Hospital
Peking University Health Science Center Hospital

Hubei Provincial Hospital of Traditional Chinese Medicine
Wuhan NO.7 Hospital
Wuhan NO.1 Hospital

Panzhihua Municipal Center for Disease Control and Prevention

Beijing Ditan Hospital, Capital Medical University

Lancang County Center for Disease Control and Prevention

Affiliated Hospital of Yangzhou University

Danyang City Center for Disease Control and Prevention

Jiangsu Provincial Center for Diseases Control and Prevention

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Shanghai Xuhui Central Hospital

Huashan Hospital affiliated to Fudan University
Beijing Ditan Hospital Capital Medical University

Anhui Provincial Hospital
Peking University First Hospital
Beijing Tongren Hospital, CMU
PetroChina Central Hospital
Hunan Provincial Center for Disease Control And Prevention

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Chang'an District Hospital of The First Affiliated Hospital of Xi'an Jiaotong University

The first people's hospital of changde city

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Province Involved in Research	Randomization	Masking
Hubei	Prior to enrollment, the researchers rechecked the selection and exclusion criteria to determine if the subjects were	Open label
Beijing	#NOTREQ	None (Open Label)
Beijing	#NOTREQ	None (Open Label)
Chongqing	Subjects included in the study after clinical diagnosis by the research leader, then the third-party statisticians use Excel to generate	Not stated
Chongqing	Subjects included in the study after clinical diagnosis by the research leader, then use Excel to generate random	Not stated
Shanghai	#NOTREQ	None (Open Label)
Hubei	Entrusting a third-party company, using a simple randomization method to generate random sequences	Not stated
Jiangsu	#NOTREQ	Quadruple (Participant, Care Provider, Investigator, Outcomes Assessor)
Guangdong	#NOTREQ	None (Open Label)

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Hubei	#NOTREQ	Quadruple (Participant, Care Provider, Investigator, Outcomes Assessor)
Beijing	#NOTREQ	Quadruple (Participant, Care Provider, Investigator, Outcomes Assessor)
Liaoning Hubei	Parallel random	Not stated
Hubei Hebei Beijing Henan Chongqing	Statistical professionals use a computer to generate a table of random numbers, and all patients are numbered by the doctor in	Not stated
Hubei Anhui Guizhou Heilongjiang Zhejiang	Randomization numbers were generated using SAS statistical software package (SAS Inc., Cary, USA). A computer generated 1:1	Open label
Hubei	The random number table method was adopted by the third party for random grouping	Not stated
Hubei	Evidence based medicine center at Tianjin University of traditional Chinese medicine will generates the random number using SAS software	Not stated
Hubei	Evidence based medicine center at Tianjin University of traditional Chinese medicine will generates the random number using SAS software	#BLANK
Hubei	SPSS was used to generate random sequences	Not stated
Hubei	SPSS was used to generate random sequences	Open label

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Hubei	SPSS was used to generate random sequences	Not stated
Hubei	SPSS was used to generate random sequences	Not stated
Hubei	SPSS was used to generate random sequences	Open label
Hubei	#NOTREQ	Single (Participant)
Hubei	#NOTREQ	Single (Participant)
Hubei	#NOTREQ	Single (Participant)
Hunan	The statistician uses SAS software to generate the random assignment table, and the researcher obtains the random number	Not stated
Zhejiang	Use stratified random method. Taking the underlying disease of organ damage as a stratification factor, a set of random	Double blind.
Zhejiang	A stratified block random method was used. Based on the 2019-nCoV pneumonia general type (early TCM clinical treatment) and heavy	Double blind
Hubei	Prior to enrollment, the researchers rechecked the selection and exclusion criteria to determine if the subjects were	open label

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Hubei	After the subjects are enrolled, random numbers are generated by software according to the block randomization method	Not stated
Zhejiang	Prior to enrollment, the researchers rechecked the selection and exclusion criteria to determine if the subjects were enrolled. SAS	Not stated
Zhejiang	Prior to enrollment, the researchers rechecked the selection and exclusion criteria to determine if the subjects were enrolled. SAS	open label
Sichuan	The randomization was implemented using the MagMinDA central random grouping system of "Xi'an Combaray Life Science	Not stated
Hubei	By trained professionals, numbered and grouped by random indicator method, in order to the patient number (according to the inclusion	Double blind.
Hubei	The R software was used to generate random Numbers at 1:1, and the generated Numbers were placed in the envelope with opaque light	Open label
Hubei	#NOTREQ	Single (Participant)
Hubei	#NOTREQ	Triple (Participant, Care Provider, Outcomes Assessor)
Hubei	The R software was used to generate random Numbers a t 1:1, and the generated Nu mbers were placed in the en velope with opaque light and	Blind method for subjects.
Zhejiang	Prior to the study, a statistician wrote the randomization protocol and, according to the randomization protocol, used	open-label

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Hubei	Random number table	single blind
Hubei Henan Guangxi Jiangsu	Non-randomization.	open label
Hubei	Subjects will be?assigned to the treatment?group and the control group on a 1:1 basis? by simple randomization. Tw o groups of random?number	N/A
Hubei	Subjects are assigned to treatment group and control group on a 1:1 basis by simple randomization. Two groups of random numbers	Open label
Zhejiang	In this trial, subjects were assigned a random number and a corresponding study drug using a central random method	Open label
Heilongjiang Hubei Anhui Jiangsu	Statistical professionals use a computer to generate a table of random numbers, and all patients are numbered by the doctor in	open label
Shanghai	#NOTREQ	None (Open Label)
Zhejiang	In this trial, subjects were assigned a random number and a corresponding study drug using a central random method	open label
Sichuan	The drugs were randomly coded.Each hospital enrolled subject was randomly coded into either the experimental or control group	Single blind
Hubei	In this study, telepnone / WeChat randomization was used. In order to ensure the concealment of distribution, the personnel responsible for	open label

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Hubei	Computer-generated sequence of random numbers	Not stated
Hubei	By trained professionals, numbered and grouped by random indicator method, the patient id number was aquired by time who was	Not stated
Guangdong	Prior to the start of the experiment, a statistical expert randomly assigns 1 to 112 numbers according to the statistical software to	Open label
Hubei	Computer software SAS to be used to generate the random number sequence, stratification by the characteristics of the	Not stated
Anhui	Random sequence is generated by the test project team's chargehand according to simple random	This test is an open test, no blind method.
Hubei	This study use randomized, open and parallel design. Whether the subject will enter into the control group or test group in the study will	open label
Hubei	Random grouping: The proc plan process of SAS9.4 software was used to generate random numbers and randomly divided into	Because Xiyanping injection and α-interferon have slight differences in appearance and color; this study uses an open design, but during the research, it is recommended that researchers who have nothing to do with the
Hubei Shanghai	The randomization method of this study is as follows: Subjects are randomly assigned to the experimental group (conventional	There is no blind method in this study.
Chongqing	SPSS was used to generate random sequences.	open label
Chongqing	In this study, a central randomization system will be used to allocate the patients to the experimental or control group at a ratio of 1:1 after	open label

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Chongqing:	The random number table generated by the PROC PLAN command in the SAS software, the automatically generated group, and the	open label
Chongqing:	The random number table generated by the PROC PLAN command in the SAS software, the automatically generated group, and the	Not stated
Hubei	Random number table was used to generate random sequences by statisticians from China Center for Evidence-Based Traditional	open label
Anhui,	The procplan program of SAS software was used to generate random numbers and randomly them divided into two groups, namely the	Not stated
Hubei	The statistician uses SAS software to generate the random assignment table, and the researcher obtains the random number	open label
Zhejiang	random number table	Not stated
Zhejiang	All patients with Novel coronavirus pneumonia were randomly divided into group A and group B by computers, and both the investigators	open label
Hubei	Random grouping method: the eligible subjects were selected from each center and consecutively grouped according to the proportion	Not stated
Hubei	The statisticians used a method of stratification and randomization by center.	open label
Hubei	The statisticians used a method of stratification and randomization by center	open label

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Beijing	Sixty patients who met the inclusion criteria were divided into a treatment group and a control group with 30 cases each	Not stated
Henan	Central random assignment method	Not stated
Shanghai	Generate 120random numbers with SPSS23.0 statistical analysis system.	Not stated
Henan	#NOTREQ	None (Open Label)
Hubei	The block random method is adopted, and the random number is generated by using the proc plan procedure statement of the	Because hydroxychloroquine and abidol hydrochloride are significantly different in appearance and color; this study uses an open design, but during the research, medicines are distributed by researchers who
Hubei	According to different research centers and whether to use stratified randomization with hormones and randomized	Not stated
Shanghai	Block randomization method will be used. The professional statistical software will be used to generate a random list of	Open label
Jiangsu	excel for randomization.	no-blinding
Hubei	The statistician uses SAS software to generate the random assignment table, and the investigator obtains the random number	Not stated
Hubei	#NOTREQ	None (Open Label)

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Zhejiang	#NOTREQ	None (Open Label)
Hubei	The statistician uses SAS software to generate the random assignment table, and the investigator obtains the random number	Not stated
Henan	Random number table	Not stated
Hubei	The procplan program of SAS software was used to generate random numbers and randomly them divided into two groups, namely the	Not stated
Henan	In this study, subjects were randomly assigned to the experimental group or control group.The randomization table was	Not stated
Zhejiang	Random number table method	Not stated
Beijing Hubei Anhui Heilongjiang Liaoning	using computer statistical software R to generate random numbers.	open label
Hubei Anhui Sichuan Henan	Statisticians generate random sequences and use randomization envelopes to randomize subjects.	Not stated
Hubei	Random grouping method: the eligible subjects were selected from each center and consecutively grouped according to the proportion	Not stated
Hubei Beijing	#NOTREQ	None (Open Label)

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Shanghai	#NOTREQ	Triple (Participant, Care Provider, Investigator)
Hubei Jiangxi Beijing	central randomization	open label
Hubei Jiangxi Beijing	central randomization	open label
Zhejiang	Professional statisticians generate random numbers by Statistical Product and Service Solutions (SPSS) 19.0	Not stated
Hubei Anhui Jiangsu Shaanxi	Some professor from Fudan University used the stratified method (taking center as stratified factor) and block random method. Using the	Not stated
Hubei Anhui Jiangsu Shaanxi	Some professor from Fudan University used the stratified method (taking center as stratified factor) and block random method. Using the	Not stated
Hubei	For patients eligible for randomization, each study center will be assigned random codes in strict order. The random code will be	Open label
Sichuan	Eligible subjects will receive a random number from small to large according to the screening number. Using the SAS9.4 software PLAN	Not stated
Hubei	Table of random numbers	Not stated
Shandong	According to the age group and clinical type of the subjects, the stratified area group of the subjects was randomly assigned to the	Open label

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Shanghai Hubei	Clinical researchers will get a random sequence number which is automatically generated by a random number generator	The trainers will not know the assessment of outcomes. To reduce the risk of bias, evaluators, data managers, and statisticians were unaware of group assignments in the outcome evaluation process and data
Hubei	Statistical experts use R software to generate random results and put them into numbered envelopes in sequence. After the	open label
Hubei	Complete random design by major researchers using computers	Not stated
Beijing	GUANGLIANG SHAN Completely random grouping	
Hubei	Randomly divided into group	Not stated
Shanghai	A as placebo group and group B as stem cell	
Jiangxi	This experiment adopts bloc	
Guangdong	k random method. Statistics personnel with the aid of stati	open label
Hubei	stical software SAS 9.4 PRO C PLAN process statements	
Hubei	The researchers randomly generated a set of random integers with a range of 2:2:1 in Excel table.	open label
Guangdong	This trial use the Central Randomization System, the	
Hubei	expert generate	open-label
Yunnan	randomization by SAS sofeware	
Hubei	Clinical researchers will get a random sequence number which is automatically generated by a random number generator	open label
Heilongjiang	Professor Li kang, Department of statistics, The first affiliated hospital of Harbin medical university, generates the random	Double blinding
Anhui	The procplan program of SAS software was used to generate random numbers and randomly them divided into two groups, namely the	Not stated

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Sichuan	cluster randomization	Not stated
Hubei	Table of random numbers	Blind method for subjects.
Guangdong Hubei	Patients were randomly coded by the sponsor, and were distributed to experimental or control groups randomly. The	Not stated
Hubei	The patients were stratified according to the respiratory support conditions and randomly assigned to the test group and the control	Not stated
Guangxi	The researchers used SPSS software to generate random numbers	Double blind
Hubei	#NOTREQ	Double (Participant, Outcomes Assessor)
Beijing	This trial will use a dynamic random approach by statisticians	open label
Hubei	Using random number table method	Open label
Anhui Hubei	Professional statisticians generate random numbers by Statistical Product and Service Solutions (SPSS) 17.0	Open label
Hubei	For patients eligible for randomization, study center will be assigned random codes in strict order. The random code will be	Not stated

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Guangdong Hubei	The statistician uses SAS software to generate the random assignment table, and the investigator obtains the random number	Double blind
Guangdong	An independent researcher used a computer-generated random number table to randomly assign eligible COVID-19 close contacts	Open-labelled
Zhejiang	Using SPSS to generate random sequences，completed by team member Yanli Wan.	Not stated
Anhui Henan Hubei Hunan Jiangxi	stratified blocked randomization	Not stated
Guangdong	#NOTREQ	None (Open Label)
Hubei	The researchers randomly generated a set of random integers with a range of 2:2:1 in Excel table.	Not stated
Guangdong Hubei Zhejiang	Clinicians in each center generate random sequences by simple randomized grouping.	Not stated
Hubei	Random number table method	Double blind method, both the doctors and the patients don't know the medicine information.
Hubei	By trained professionals, numbered and grouped by random indicator method, in order to the patient number (according to the inclusion	open label
Hubei Chongqing Shanxi	Resman (www.medresman.ORG), a public management platform for clinical trial management of China clinical trial registration	Not stated

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Beijing	The third party USES computer software to generate the random distribution table randomly, and the third party is	Not stated
Shanghai	The procplan program of SAS software was used to generate random numbers and randomly them divided into two groups, namely the	Not stated
Tianjin Hubei	Randomized parallel trial	Not stated
Jiangxi	Generated by a table of random numbers	Not stated
Shanghai	A stratified block randomization method was used. The stratified variables were age and whether antiviral drugs had been	Open label
Hubei	The RAND function in Excel was used to generate random sequences. The random sequences were put into opaque kraft envelopes	Not stated
Hubei Shaanxi	Randomized grouping，random sequence table.	Not stated
Hainan	In order to reduce possible selective bias and curative effect bias from researchers and subjects, this study will use a randomized, double-	Double blind
Beijing	Before subjects were enrolled, researchers used SPSS 24.0 software to generate random Numbers and serial Numbers, and	Open label, Non-blinding.
Hubei	Block randomization method will be used. The professional statistical software will be used to generate a random list of	Not stated

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Hunan	random number table	Not stated
Jiangxi	Random number table	Not stated
Hubei	Random number table generated by professional statistical software	Not stated
Hubei	EBM & Clinical Research Service Group of Guangdong Provincial Hospital of Chinese Medicine will generates the random	Not stated
Hubei	#NOTREQ	Quadruple (Participant, Care Provider, Investigator, Outcomes Assessor)
Hubei	#NOTREQ	Triple (Participant, Care Provider, Outcomes Assessor)
Sichuan	SPSS was used to generate random sequences.	Not stated
Jiangxi	Not stated	Not stated
Jiangxi	Random grouping using SAS9.4 software	#BLANK
Hubei	The project leader generates random sequences from a table of random digit.	Not stated

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Hubei	Random number is from random number table	Open label
Hubei	SAS9.4 generated the random number sequence.	Not stated
Hubei	parallel	Not stated
Shanghai	Parallel study	Not stated
Sichuan	The experimenters were divided into groups by random number table. After each patient was randomly arranged and numbered, the	Open label
Jiangsu	Random envelope method	Not stated
Heilongjiang	In this study, patients with high inflammation will be randomly divided into two groups according to the ratio of 1: 1. The length of the	Not stated
Hubei	SAS 9.4 software was used to generate random sequences by statisticians from China Center for Evidence-Based Traditional	open label
Hubei	According to the above included diagnosis of severe and critical patients, and the new coronavirus positive 200 patients, patients will be	Not stated
Sichuan	The research assistants who were not involved in case inclusion first used excel to generate 300 random Numbers (according to	Open label

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Hubei	A biostatistician who has no relationship with the statistical analysis of data management in this experiment, using a SAS	Not stated
Shaanxi	The researchers used a dice roll to randomly assign the enrolled patients.	Open label
Shanghai	#NOTREQ	Triple (Participant, Care Provider, Investigator)
Hubei	In this study, cases are randomly assigned to experimental and control groups at a ratio of 1: 1 by block randomization	Open label
Hubei	Random number table method	Not stated
Hubei	The IWRS random system is adopted, and the hierarchical block randomization is adopted, and the group is obtained in chronological	Double blind
Heilongjiang	The random number table used in this test plan was generated by biostatistics professionals using professional statistical	Not stated
Fujian	Random number table	Open label
Hubei	The third party statisticians used SAS software to generate random sequences using the method of stratified randomization	Open label
Fujian	80 clinical first-line nurses were selected. The subjects were divided into experimental group (40) and control group (40) by parallel	Open label

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Hubei	Computer-generated sequence of random numbers	Open label
Hubei	The third party statisticians used SAS software to generate random sequences using the method of stratified randomization	Open label
Hubei	A random number table is generated by the computer, and the subjects are numbered sequentially according to the order of the	#BLANK
Guangdong Hubei	Recruited patients were randomly assigned to each hospital by telephone verification in a ratio of 1:1. Four patients in each district	Single blind
Hubei	Stratified block randomization	Open label
Jiangsu Hubei	The patients who meet the inclusion criteria and do not meet the exclusion criteria will be sorted according to the order of admission, and	Open label
Jiangsu Hubei	The patients who meet the inclusion criteria and do not meet the exclusion criteria will be sorted according to the order of admission, and	open label
Hubei	Central randomization was used in this study. In order to ensure the concealment of distribution, personnel responsible for	Open label
Hubei	The researchers number the qualified subjects according to the order of their visits, and divide them into the corticosteroid therapy group	Not stated
Hubei	This study is a prospective randomized clinical trial using random stratification.Enrolled patients were randomly divided into two groups.The contr	#BLANK

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Hubei	In this experiment, the simple random grouping method was used.	#BLANK
Zhejiang	#NOTREQ	None (Open Label)
Heilongjiang	Statistical professionals use a computer to generate a table of random numbers, and all patients are numbered by the doctor in	open label
Zhejiang	Statistical professionals use a computer to generate a table of random numbers, and all patients are numbered by the doctor in	Not stated
Hubei Sichuan	SAS was used to generate random sequences. Each hospital enrolls subject will be randomly coded into either the experimental or	Double blind
Hubei	A multicenter, randomized, parallel controlled trial design was used in this study.The research in each research center is random The study	Blind method for subjects.
Hubei	The researchers number the qualified subjects according to the order of their visits, and divide them into the corticosteroid therapy group	Not stated
Hubei	For patients eligible for randomization, each study center will be assigned random codes in strict order. The random code will be	Not stated
Shandong	The clinicians of this study used the EXCEL table to generate random sequences before patients were enrolled, and were divided	Open label
Hubei	Computer random number	#BLANK

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Henan	Random number table	Not stated
Beijing Hubei	#NOTREQ	None (Open Label)
Shandong	Use Excel to generate random Numbers to generate random sequences	N/A
Shandong	#NOTREQ	Triple (Participant, Care Provider, Outcomes Assessor)
Hubei	This trial will use a group random approach by authorized staff.	open label
Guangdong	This trial will use a group random approach by authorized staff.	Open label
Hubei	The randomization method of this study is as follows: Subjects are randomly assigned to the experimental group (conventional	There is no blind method in this study.
Hunan	Centralized randomization, using the eLite stochastic system of Nanjing Medical University, randomly assigned patients to the	The primary outcome measures CT results were assessed by a third-party blind method outside the research team
Hubei	doctor decides that by envelope method	Not stated
Hubei	Statistical experts use R software to generate random results and put them into numbered envelopes in sequence. After the	open label

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Hubei	Subjects will be assigned to the western medicine treatment group and the TCM treatment group on a 1:1 basis by simple	N/A
Guangdong	Random Numbers Generated by Random Functions	Not stated
Zhejiang	The patients were randomly allocated on a 1:4 ratio((control group:experimental group)) to receive by a computer-	#BLANK
Shanghai Hubei	The procplan program of SAS software was used to generate random numbers and randomly them divided into two groups namely the	Not stated
Guangdong	The research center conducted computer-generated block randomization (block size =8)	Not stated
Hubei	#NOTREQ	Quadruple (Participant, Care Provider, Investigator, Outcomes Assessor)
Guangdong	The Doll's clinical case random table was adopted by the special scientific research assistant, and the method was as follows: the	none
Zhejiang	This study plans to collect 20 subjects, according to the order of the study subjects into the group numbering, using statistical software to	Not stated
Zhejiang	This study plans to collect 100 subjects, according to the order of the study subjects into the group numbering, using statistical	Not stated
Guangdong	Envelope Random Method by Attending Doctor	Not stated

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Shaanxi	The statistician uses the SAS software proc plan program to generate the random sequence.	Open-label
Hubei	The subjects were stratified according to the respiratory support conditions, and then randomly assigned to the test group and the control	It is a double-blind study. Subjects, researchers (all participants in this study, including but not limited to those who are responsible for screening subjects, follow-up and evaluating outcome and compliance with
Jiangxi	A specific researcher is responsible for generating a random sequence using the statistical software SAS 9.1	Open label
Guangdong Henan Sichuan Heilongjiang Shandong	program. Eligible patients Use the central randomization system	Open label
Jiangxi	Randomly enter a group according to the time sequence of subjects' entering the group and the pre-determined random	Open label
Beijing	#NOTREQ	Double (Participant, Care Provider)
Hubei	A static random sequence will be formed by GPHCM and placed on a clinical information collection platform to apply for the	Open label
Hubei	Random numbers generated by computers	Not stated
Hubei	Patients were randomly divided into two groups by the nurse's random envelope method	Open label
Hunan	Subjects were grouped by the random sequence generated according to the random number table by the staff of the Clinical Trial	NA

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Shanghai	Stratified block randomization	Not stated
Guangdong	#NOTREQ	Single (Participant)
Beijing	The researchers in the project team used EXCEL to generate a table of random Numbers, which were divided into two groups	Single blind
Guangdong	#NOTREQ	Quadruple (Participant, Care Provider, Investigator, Outcomes Assessor)
Chongqing	#NOTREQ	Quadruple (Participant, Care Provider, Investigator, Outcomes Assessor)
Fujian	#NOTREQ	None (Open Label)
Guangdong	SAS software will be used to generate random number by stratified block randomization and the length of block will be 2 or 4	Not stated
Hubei	Third parties that do not participate in the study use computer software Excel to generate random sequences and custody	Not stated
Hubei	#NOTREQ	Triple (Participant, Investigator, Outcomes Assessor)
Hubei	Eligible patients were randomly assigned to the Nintedanib group and the control group	Not stated

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Guangdong	SPSS25.0 was used to generate 56 random Numbers in the range of 1-100 (fixed number of seed is 200 000)	Double blind
Anhui Hubei Zhejiang	#NOTREQ	None (Open Label)
Anhui Hubei Zhejiang	#NOTREQ	None (Open Label)
Hubei	Random number table method	Open label
Hubei	The third party uses statistical software to generate random number.	Not stated
Hubei	#NOTREQ	Double (Participant, Investigator)
Zhejiang	The computer assigns random Numbers	Not stated
Henan	Stata 12.0 software will be used to generate random codes by randomization method, and the test vaccine and the control vaccine will	Double blind
Jiangsu Hubei	Meng hongdao, a data statistician, used the software to prepare a random number table, which was used to divide subjects	Open label
Guangdong	Randomized parallel control （Grouped by a computer table of random numbers）	Not stated

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Jiangsu	#NOTREQ	Quadruple (Participant, Care Provider, Investigator, Outcomes Assessor)
Hubei	SAS 9.4 software was used to generate random sequences by statisticians from China Center for Evidence-Based Traditional	Not stated
Hubei	The researchers did randomize the volunteers.Cluster randomization was used in Huangshang Group Simple	#BLANK
Hubei	The researchers used a random number table to randomly divide the patients into three groups, namely the control group, the 1.5ΔTA	Not stated
Shanghai	The researchers in the project team used EXCEL to generate a table of random Numbers, which were divided into two groups	Not stated
Hubei	#NOTREQ	Single (Outcomes Assessor)
Hubei	Centralized block randomization method was accessed.Subjects are assigned to treatment group and control group on a 1:1	Double blind
Hubei	Implementation of the randomized method through the central randomized system of clinical research	Double blind
Hubei	SAS software was used to randomize the grouping and generate a random number table	Not stated
Heilongjiang	The third party uses a random number table for randomization	Open label

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Shaanxi	#NOTREQ	None (Open Label)
Beijing	Using block randomization, all selected cases are divided into test group and control group, and the block size is 6. Using SAS	Not stated
Sichuan Shanghai Hubei	The random number generated by a computer.	Not stated
Shanghai Hubei	The statistician applies a random sequence number automatically generated by a random number generator (IBM Corp., Armonk, NY	Participants and Liu-zi-jue trainers are unable to be blind about group assignments due to the specific intervention, but the trainers will not know the assessment of outcomes. To reduce the risk of bias, evaluators, data
Hubei	SAS software was used to randomize the grouping and generate a random number table	#BLANK
Hubei	The random sequence will be generated by computer.	Not stated
Henan	Using Stata 12.0 software to generate random codes by randomization, the study vaccine and the placebo were randomly assigned to	Not stated
Hubei	stratified block randomization	Not stated
Guangdong Hubei InnerMongolia	A randomization scheme that generates a list of 160-case randomization sequence utilizing the stratified block randomization method by	double-blind
Hubei	Block randomization was adopted, and the time of admission was used as the compatibility factor, block size is four, by a ratio of 1:1	Double blind

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Guangdong	The enrolled patients will be allocated into routine treatment group and WMT group by the block randomization method	Not stated
Shangnai Guangdong Tianjin Heilongjiang Beijing	In this study, disease severity (mild, moderate, severe) and age (< 60 years old, ≥60 years old) were stratified and randomized	#BLANK
Hebei	#NOTREQ	Double (Participant, Investigator)
Jiangsu	#NOTREQ	Double (Participant, Outcomes Assessor)
Heilongjiang	Randomly grouped by the system	Not stated
Hubei	Statistical professionals generate random number tables according to the clinical research central random system (network	Double blind
Liaoning Beijing Hunan	Randomized with envelopes	Not stated
Zhejiang	Statistical professionals use a computer to generate a table of random numbers, and all patients are numbered by the doctor in	Not stated
Sichuan	#NOTREQ	Quadruple (Participant, Care Provider, Investigator, Outcomes Assessor)
Hubei Jiangsu	Random allocations are generated by the use of an Excel random number table at the design stage and are based on the calculated	Open label

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Guangdong	The random program was provided by the statistics professionals of the school of public health of Fudan University. Stratified (with the	Not stated
Hubei	This study uses the interactive web response system(IWRS) to random,in order to ensure that the allocation is hidden the	Double blind
Shanghai Hubei	The statistician applies a random sequence number automatically generated by a random number generator (IBM Corp., Armonk, NY	Participants and Liu-zi-jue trainers are unable to be blind about group assignments due to the specific intervention, but the trainers will not know the assessment of outcomes. To reduce the risk of bias, evaluators, data
Shanghai	Use the block random method to formulate a randomization plan.	#BLANK
Shanghai	#NOTREQ	Quadruple (Participant, Care Provider, Investigator, Outcomes Assessor)
Zhejiang	The third party uses a random number table for randomization	Not stated
Chongqing	#NOTREQ	Double (Participant, Investigator)
Anhui	#NOTREQ	#NOTREQ
Zhejiang Guangxi	SAS statistical software (version 9.4) is used by an unblinded randomization statistician to generate a randomized blinding code	Not stated
Hunan	#NOTREQ	Double (Participant, Investigator)

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Sichuan	#NOTREQ	Quadruple (Participant, Care Provider, Investigator, Outcomes Assessor)
Beijing	#NOTREQ	Single (Participant)
Beijing	#NOTREQ	Single (Participant)
Beijing	#NOTREQ	Triple (Participant, Care Provider, Investigator)
Hubei	With the aid of SAS9.3 statistical analysis system, a randomized plan of subjects was generated, and the ratio of patients in the two groups	Doubel blinding
Shanghai	Random Numbers generated by statistical software	Double blind
Hunan	#NOTREQ	Quadruple (Participant, Care Provider, Investigator, Outcomes Assessor)
Jiangsu	#NOTREQ	Triple (Participant, Investigator, Outcomes Assessor)
Shanghai	#NOTREQ	Quadruple (Participant, Care Provider, Investigator, Outcomes Assessor)
Jiangsu	#NOTREQ	Triple (Participant, Care Provider, Investigator)

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Hubei	Use the statistical software to generate a random number table	#BLANK
Jiangsu	A block random design is adopted. Each random block contains several random numbers, and each random number corresponds to three	Double blind
Shanghai	The statistician will assign codes according to the order in which the subjects are enrolled.	Double blind
Hebei	#NOTREQ	Double (Participant, Investigator)
Hubei Beijing	Adopting the clinical experiment management method of central randomization system, by dialing the telephone number	#BLANK
Hubei Beijing	Using center randomized clinical trial management system, the method of by call the randomization center (interactive voice response	Not stated
Jiangsu	#NOTREQ	Quadruple (Participant, Care Provider, Investigator, Outcomes Assessor)
Beijing Shanghai	#NOTREQ	None (Open Label)
Hubei	In this experiment, the random distribution table of cases was compiled by spss22.0 statistical software. according to the random	Not stated
Jiangsu	Randomized statisticians use SAS version 9.4 or above to generate random blinding code. In each age and dose group, subjects were divided	Double blind

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Guangdong	#NOTREQ	Quadruple (Participant, Care Provider, Investigator, Outcomes Assessor)
Zhejiang	SAS statistical software (version 9.4) is used by an unblinded randomization statistician to generate a randomized blinding code	Double blind
Hubei	The group of patients will be determined by drawing lots from the nurse in charge	Not stated
Jiangsu	Randomized statisticians use SAS version 9.4 or above to generate random blinding code. In each age and dose group, subjects were divided	Double blind
Jiangsu	#NOTREQ	Double (Participant, Investigator)
Jiangsu	SAS 9.4 software is used to carry out randomization according to the stratified block randomization method.	Double blind
Shanghai	#NOTREQ	Quadruple (Participant, Care Provider, Investigator, Outcomes Assessor)
Jiangsu	#NOTREQ	Quadruple (Participant, Care Provider, Investigator, Outcomes Assessor)
Jiangsu	#NOTREQ	Triple (Participant, Care Provider, Investigator)
Jiangsu	#NOTREQ	Triple (Participant, Care Provider, Investigator)

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Jiangsu	The randomization program is completed by the randomization team.	Double blind
Hunan	This trial adopts a stratified block randomization method, stratified by center (participating country), age (18-59 years old, 60 years	This study adopts a double-blind design. Randomization statisticians and other blind-editing personnel will perform vaccine blinding, that is, paste the printed vaccine label on the designated position of each
Shaanxi	#NOTREQ	Single (Participant)
Shanghai	Computer software produces random sequences	Not stated
Beijing	#NOTREQ	Single (Participant)
Shanxi	Randomize according to the cluster random method with SAS 9.4 software.	Not stated
Shanxi	Randomize according to the block random method with SAS 9.4 software.	Not stated
Shanxi	Randomize according to the block random method with SAS 9.4 software.	Not stated
Guangxi Shanxi	SAS statistical software (version 9.4) is used by an unblinded randomization statistician to generate a randomized blinding code	Blind method for subjects.
Hebei Heilongjiang	Statistical professionals use a computer to generate a table of random numbers, and all patients are numbered by the doctor in	Open-label

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Hebei Heilongjiang	Statistical professionals use a computer to generate a table of random numbers, and all patients are numbered by the doctor in	Open-label
Hebei Heilongjiang	Statistical professionals use a computer to generate a table of random numbers, and all patients are numbered by the doctor in	Not stated
Zhejiang	In this study, a randomized block method was used, with each subject assigned to the trial group or placebo group determined by a randomized	Double blind
Jiangsu	#NOTREQ	Triple (Participant, Care Provider, Investigator)
Hebei	The statistical staff at evidence-based medicine center of Tianjin University of traditional Chinese medicine used SAS software to	Not stated
Hebei	Statistical professionals use a computer to generate a table of random numbers, and all patients are numbered by the doctor in	Not stated
Hubei	Generated by Computer software	Double blind
Beijing Guangdong Jiangsu Yunnan	#NOTREQ	Single (Participant)
Shanghai	SAS software will be used to generate random codes by randomization method. The ratio of experimental group and control group is 1:1	Open-label
Zhejiang	#NOTREQ	None (Open Label)

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Jiangsu	#NOTREQ	Triple (Participant, Care Provider, Investigator)
Sichuan	Random number table	Not stated
Jiangsu	#NOTREQ	Triple (Participant, Investigator, Outcomes Assessor)
Guangdong	For the planned 2-dose and 1-dose youth (18-59 years) and elder (>=60 years) groups, block randomization is performed by the	Double blind
Guangdong	As a dose-escalating study, it is planned to adopt six groups according to age (youth and elder) and dosage (10 ug, 25 ug and	Double blind
Guangdong	The randomization will be carried out in the Clinical Trail Unit of the First Affiliated Hospital, Sun Yat-sen University. The subjects	N/A
Jiangsu	#NOTREQ	Quadruple (Participant, Care Provider, Investigator, Outcomes Assessor)
Henan	#NOTREQ	Double (Participant, Investigator)
Henan	#NOTREQ	None (Open Label)
Zhejiang	Randomized blind bottom was generated by block randomization using SAS statistical software (version 9.4 or above) by randomized	Double blind

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Hebei	#NOTREQ	Double (Participant, Investigator)
Shanghai Sichuan Shanxi	The researchers use SAS software for block randomization	Not stated
Hubei Henan Guangdong	The researchers use SAS software for block randomization	Not stated
Hebei	Central block randomization method	Not stated
Liaoning	#NOTREQ	Double (Investigator, Outcomes Assessor)
Jiangsu	#NOTREQ	Quadruple (Participant, Care Provider, Investigator, Outcomes Assessor)
Jiangsu	#NOTREQ	Triple (Participant, Investigator, Outcomes Assessor)
Jiangsu	SAS 9.4 software is used to carry out randomization according to the stratified block randomization method.	Not stated
Shanghai	SAS software will be used to generate random codes by randomization method. The length of blockl is 10	Not stated
Guangdong	The central random method is used to intervene according to the allocation plan. The treatment plan received by each research	Not stated

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Shanxi Sichuan	#NOTREQ	Quadruple (Participant, Care Provider, Investigator, Outcomes Assessor)
Anhui	#NOTREQ	Triple (Participant, Care Provider, Investigator)
Shandong	#NOTREQ	None (Open Label)
Hu'nan	#NOTREQ	Quadruple (Participant, Care Provider, Investigator, Outcomes Assessor)
He'nan	#NOTREQ	Triple (Participant, Investigator, Outcomes Assessor)
He'nan	#NOTREQ	Triple (Participant, Investigator, Outcomes Assessor)
Jiangsu	#NOTREQ	Double (Participant, Investigator)
Hu'nan	#NOTREQ	Double (Participant, Investigator)
Hu'nan	#NOTREQ	Double (Participant, Investigator)
Jiangsu	#NOTREQ	Double (Participant, Investigator)

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Jiangsu	#NOTREQ	Triple (Participant, Care Provider, Investigator)
Hu'nan	The method of randomization and blinding was realized by vaccine editing, and the random table was generated by SAS	Not stated
Hubei Jiangsu	Random Stratified random number table	Not stated
Jiangsu	The clinical research methodology team of Guangdong Hospital of traditional Chinese medicine generated random	Open-label
Jiangsu	The central randomization method is used to intervene according to the allocation plan. The treatment plan received by each research	Not stated
Jiangsu	SAS 9.4 software was used to implement the cluster randomization by statisticians from China Center for Evidence-Based	Open label
Yunnan	Randomization codes will be generated by an unblinded statistician responsible for randomization using SAS statistical software (version	Not stated
Shanxi	Randomization codes will be generated by an unblinded statistitian responsible for randomization using SAS statistical software (version	Not stated
Chongqing	Randomization list will have been generated by a Statistician using a computer and will be properly kept by a Designated Study	#BLANK
Zhejiang	The randomization statistician used SAS statistical software (version 9.4 or above) to randomize each dose group of the adult	Not stated

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Guangxi Yunnan	Non-blind randomization statisticians use SAS statistical software (version 9.4) to generate randomized blind bottoms using block	Not stated
Hubei	SAS was used to generate random sequences	Not stated
Shanghai	#NOTREQ	None (Open Label)
Shanghai	#NOTREQ	None (Open Label)
Hubei	Clinical researchers will get a random sequence number which is automatically generated by a random number generator	Not stated
Jiangsu	#NOTREQ	None (Open Label)
Hubei	SAS was used to generate random sequences	Not stated
Jiangsu	#NOTREQ	Triple (Participant, Investigator, Outcomes Assessor)
Beijing	#NOTREQ	None (Open Label)
Guangdong He'nan Hubei	#NOTREQ	Single (Outcomes Assessor)

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Hebei Heilongjiang	#NOTREQ	Single (Participant)
Jiangsu	#NOTREQ	Double (Participant, Investigator)
Hu'nan	#NOTREQ	Quadruple (Participant, Care Provider, Investigator, Outcomes Assessor)
Hubei	Dr. Le Shengjong, a member of the project team, divided the subjects into two groups (20 in each group) according to the random numbers	Not stated
Hubei	Random number table method.	Open label
Hu'nan	Random and blind methods were realized through vaccine coding, and SAS software was used to generate random tables. The	Double blind
Beijing	The statistician uses the random block method to generate random sequences.	Not stated
He'nan	In this study, subjects were randomly assigned to the experimental or control group.The randomization table was randomly	Open-label
Shanghai	The statistician used SPSS statistical software to randomly assign 50 numbers into 2 groups by randomization	Not stated
Hu'nan	#NOTREQ	None (Open Label)

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Guangxi	The randomization codes will be generated by the randomization statistician using the block randomization method	Not stated
Guangdong	#NOTREQ	None (Open Label)
Sichuan	The randomization program is completed by the randomization team.	Double blind
Shanghai	This study is an open randomized controlled study design. Professional medical statisticians use SAS?9.4 statistical software to	Open-label
Shanxi	#NOTREQ	Double (Participant, Investigator)
Jiangsu	#NOTREQ	Triple (Participant, Investigator, Outcomes Assessor)
Guizhou	#NOTREQ	None (Open Label)
Beijing	#NOTREQ	Double (Participant, Investigator)
Guangdong	#NOTREQ	Double (Participant, Investigator)
Yunnan	Non-blind randomization statisticians use SAS statistical software (version 9.4) to generate randomized blind bottoms using block	Not stated

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Shanghai	#NOTREQ	Double (Participant, Investigator)
Beijing	#NOTREQ	Triple (Participant, Investigator, Outcomes Assessor)
Beijing	#NOTREQ	Triple (Participant, Investigator, Outcomes Assessor)
Hubei	The patients were randomly divided into two groups by the central random system at a 1:1 ratio.	Not stated
Sichuan	The trial used a block randomization method, stratified by center and age (18-59 years, ≥60 years). The randomization of	#BLANK
Beijing	Central Random System Block Random	Not stated
Yunnan	The randomization codes will be generated by the randomization statistician using the block randomization method	Not stated
Jiangsu	This study was divided into intervention group and control group by random number method. Patients with COVID-19 were	#BLANK
Jiangsu	The randomization program is completed by the randomization team	Not stated
Jiangsu	#NOTREQ	None (Open Label)

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Shanghai	#NOTREQ	Triple (Participant, Care Provider, Investigator)
Shanghai Beijing	#NOTREQ	None (Open Label)
Anhui Beijing Hebei Hu'nan	#NOTREQ	Double (Participant, Investigator)
Hebei Beijing Hunan Chongqing Fujian	#NOTREQ	Triple (Participant, Investigator, Outcomes Assessor)
Shaanxi	A researcher used a random number table method to generate a random sequence, with odd numbers entering the control group	#BLANK
Hunan	Generate random sequences with SPSS software.	#BLANK

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Participants (Patient or Suspected/Close Contacts or Healthy/Other)
Patient
Patient
Patient
Patient
Patient
Patient
Patient
Patient
Patient

For peer review only

Patient

Patient

Patient

Suspected/Close Contacts

Patient

Suspected/Close Contacts

Patient

Patient

Healthy/Other

Healthy/Other

For peer review only

Patient

Healthy/Other

Healthy/Other

Patient

Patient

Patient

Patient

Suspected/Close Contacts

Patient

Patient

For peer review only

Patient

Patient

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For peer review only

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Suspected/Close Contacts

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For peer review only

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For peer review only

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Inclusion Criteria

Inclusion criteria: 1. Adult aged >=18years old;
2. Patients infected with new coronavirus confirmed by PCR
3. In the state of no oxygen at rest, the patient's SPO2<=94% or the oxygenation index is less than 300mmHg.

Inclusion Criteria:

Adult
PCR confirmed COVID-19 infection
Symptoms developed more than 7 days

Inclusion Criteria:

Confirmed COVID-19 infection case in term of laboratory evidence;
80 years ≥ age ≥ 14years;
Within 72 hours after the onset of abnormalities shown by Chest radiology or several symptoms

Inclusion criteria: 1. Male or female over 18 years of age;
2. Novel coronavirus infection is confirmed by pathogenic detection;
3. The diagnosis of severe coronavirus pneumonia will have to meet at least one of the following criteria:
(1) Respiratory distress, RR>30 times/minute
(2) In the state of no oxygen at rest, the patient's SPO2<=93%
(3) Oxygenation Index (PaO2/FiO2)<=300 mmHg (1mmHg=0.133kPa);
(4) Respiratory failure requiring mechanical

Inclusion criteria: (1) 18â€“65 years of age;
(2) Diagnosed as mild to moderate COVID-19;
(3) Be willing to sign informed consent;
(4) The clinical assessment and condition of the subject does not influence the completion of the study.

Inclusion Criteria:

The participants were diagnosed as COVID-19 pneumonia, according to the notice on printing and distributing the diagnosis and treatment plan of pneumonia with new coronavirus infection (trial version 4 or update version) made by National Health Commission of the People's Republic of

Inclusion criteria: 1) Aged >= 18 years;
2) Meet the diagnostic criteria of severe pneumonia patients with new coronavirus;
3) The informed consent form.

Inclusion Criteria:

14-70 years old
2019 novel coronavirus pneumonia (severe type)
Subjects voluntarily participate in the clinical trial and sign the informed consent

Inclusion Criteria:

In sputum, throat swab, lower respiratory tract secretion, blood and other samples, the nucleic acid of the novel coronavirus was positive, or the sequencing of the virus gene was highly homologous with the known novel coronavirus

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Inclusion Criteria:

Age ≥18 years at time of signing Informed Consent Form
Laboratory (RT-PCR) confirmed COVID-19.
Lung involvement confirmed with chest imaging

Inclusion Criteria:

Age ≥18 years at time of signing Informed Consent Form
Laboratory (RT-PCR) confirmed COVID-19.
Lung involvement confirmed with chest imaging

Inclusion criteria: 1. 2019-nCoV infected patients treated at Affiliated Zhongshan Hospital of Dalian University and Zhongnan Hospital of Wuhan University from January 2020 to December 2020
2. nCoV nucleic acid positive;
3. Aged >=18 years;
4. Patients understand the purpose of the trial, voluntarily sign informed consent, and agree to complete the rectification research process

Inclusion criteria: 1. Those who meet the diagnostic criteria for suspected cases in the diagnosis and treatment scheme;
2. Over 18 years old (including 18 years old), male or female;
3. At the time of enrollment, the patient had any of the main symptoms (fever, cough, fatigue);
4. Voluntarily sign a written informed consent before the study begins.

1. Patients with confirmed new coronavirus-infected pneumonia;
2. Above 18 years old (inclusive);
3. At the time of enrollment, the patient had any of the main symptoms (fever, cough, fatigue);
4. Voluntarily sign written informed consent.

Inclusion criteria: 1. aged 18 to 60 years
2. to be confirmed with a exposure history of diagnosed population within last 14 days;
3. with fever, positive results of blood routine examination and positive findings in CT examination, any two in three items met.

Inclusion criteria: 1) Pneumonia patients diagnosed as new coronavirus infection;
2) The type of pneumonia was judged as severe and critical;
3) Informed consent.

Inclusion criteria: 1) patients diagnosed as Pneumonia caused by new coronavirus infection (2019-nCoV);
2) The classification was judged as common type;
3) Informed consent.

Inclusion criteria: 1. patients diagnosed as Pneumonia caused by new coronavirus infection (2019-nCoV);

2. virus turned negative after treatment;

3. aged 18-70 years old;

4. patients with clear consciousness;

Inclusion criteria: 1. patients diagnosed as Pneumonia caused by new coronavirus infection (2019-nCoV);
2. virus turned negative after treatment;
3. aged 18-70 years old;
4. patients with clear consciousness
5. Signing the informed consent form.

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Inclusion criteria: 1. patients diagnosed as Pneumonia caused by new coronavirus infection (2019-nCoV);The classification was judged as common type;
2. aged 18-70 years old;
3. patients with clear consciousness;
4. Signing the informed consent form.

Inclusion criteria: 1. patients diagnosed as Pneumonia caused by new coronavirus infection (2019-nCoV);
2. virus turned negative after treatment;
3. Patients diagnosed with pulmonary fibrosis;
4. aged 18-70 years old;
5. patients with clear consciousness;
6. Signing the informed consent form.

Inclusion criteria: 1. patients diagnosed as Pneumonia caused by new coronavirus infection (2019-nCoV);
2. virus turned negative after treatment;
3. aged 18-70 years old;
4. patients with clear consciousness
5. Signing the informed consent form.

Inclusion Criteria:

1)2019-nCoV nucleic acid test was positive. 2)CT of the lung conformed to the manifestation of viral pneumonia.

Inclusion Criteria:

2019-nCoV nucleic acid test was positive.
CT of the lung conformed to the manifestation of viral pneumonia.

Inclusion Criteria:

1.Meet the definition of severe pneumonia(Comply with any of the followings):

Shortness of breath RR>30 bpm;

Inclusion criteria: (1) aged 18 to 70 years;
(2) Light and heavy patients with new coronavirus infection confirmed by etiology.

Inclusion criteria: (1) Conforming to the diagnostic criteria of suspected cases (Chiese medicine observation period) in the fourth edition of pneumonia diagnosis and treatment program for new coronavirus infection issued by the national health and health commission;
(2) Age between 14 and 80 years old;
(3) Patients informed and agreed to participate in the study.

Inclusion criteria: (1) Patients who meet the diagnostic criteria of the common type of confirmed cases (the initial stage of clinical treatment of traditional Chinese medicine) or the severe cases of confirmed cases (the middle stage of clinical treatment of traditional Chinese medicine);
(2) The cases of common type (the initial stage of clinical treatment of traditional Chinese medicine) belong to the type of cold-dampness stagnant lung;
(3) The confirmed cases were severe (in the

Inclusion criteria: 1. Adult aged >= 18 years old;
2. Patients with unexplained viral pneumonia that have been clinically diagnosed or patients with novel coronavirus serum antibody (IgM or IgG) positive or patients with novel coronavirus infection confirmed by PCR when no rapid diagnostic kit has been developed;
3. The interval between the onset of symptoms and randomized is within 7 days. The onset of symptoms is mainly based on fever. If there is no fever, cough or other related

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Inclusion criteria: 1. Aged 18-65 years;
2. been Confirmed with 2019-nCoV pneumonia, hospitalized or will be hospitalized;
3. Sign the ICF;
4. no more than 10 days from the occurrence of relevant clinical symptoms to the diagnosis been confirmed.

Inclusion criteria: 1. 18 to 75 years of age, male or female, willing to sign informed consent;
2. Tested positive for novel coronavirus infection after the onset of symptoms using a real time polymerase chain reaction (RT-PCR)-based diagnostic assay;
3. At least 2 viral nucleic acid tests are still positive under current antiviral therapy;
4. No difficulty in swallowing the pills;
5. Willing to abide by the protocol

Inclusion criteria: 1. Aged 18 to 75 years male or female, willing to sign the informed consent;
2. Tested positive for novel coronavirus infection after the onset of symptoms using a real time polymerase chain
reaction (RT-PCR)-based diagnostic assay;
3. Enrollment and initiation of study drug treatment <=96 hours after onset of symptoms;
4. No difficulty in swallowing the pills;
5. Willing to abide by the protocol

Inclusion criteria: Patients with suspected and confirmed diagnosis of 2019-nCoV Pneumonia Version 4.0 (published at 2020.01.27) of the National Health Commission

Inclusion criteria: Patients with novel coronavirus pneumonia who agreed to participate in this trial and signed the informed consent form.

Inclusion criteria: On January 27, 2020, the general office of the national health commission and the office of the state administration of traditional Chinese medicine issued the pneumonia diagnosis and treatment plan for new coronavirus infection (trial version 4).
1. patients with severe 2019-ncov according to the clinical stage met any of the following criteria:
(1) Respiratory distress, RR>=30 times/min;
(2) In resting state, oxygen saturation is less than 93%;
(3) Partial

Inclusion Criteria:

Age:18~55 years old, unlimited gender.
Laboratory (RT-PCR) confirmed infection with 2019-nCoV and accompanied with clinical manifestations

Inclusion Criteria:

≥ 18 years old;
Diagnosed as serious or critical SARI (according to the 4th version of Diagnosis and Clinical management of 2019-nCoV infected pneumonia);

Inclusion criteria: On January 27, 2020, the general office of the national health commission and the office of the state administration of traditional Chinese medicine issued the pneumonia diagnosis and treatment plan for new coronavirus infection (trial version 4).
Only patients with severe and critical 2019-ncov were included in this study.
1. Patients with severe 2019-ncov according to the clinical stage met any of the following criteria:
(1) Respiratory distress, RR >= 30 times/min;

Inclusion criteria: o be enrolled, all the following criteria must be met:
1) the confirmed patient (or legal guardian) agrees to participate in the study and signs the informed consent;
2) Aged 18-65 years, gender not limited;
3) the time interval between symptom onset and random enrollment was within 8 days (symptom onset was mainly determined by the time of fever);
4) real-time fluorescence rt-pcr detection of respiratory tract or blood samples in patients with positive

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Inclusion criteria: 1. Aged >= 18 years and <= 75 years;
2. Patients clinically diagnosed as novel coronavirus infection; or with positive serum antibodies (IgM or IgG); or with novel coronavirus infection confirmed by PCR;
3. Severe/critical patients.

Inclusion criteria: (1) Comply with the diagnostic criteria of "Pneumonitis Diagnosis and Treatment Scheme for New Coronavirus Infection (Trial Version 5)";
(2) The classification was judged as common and severe type;
(3) Having one or more symptoms of fever, cough and fatigue when entering the group;
(4) The diagnosis time does not exceed 48 hours;
(5) Aged >= 18 years

Inclusion criteria: 1. Suspected and confirmed cases of pneumonia in accordance with the above new signs of coronavirus infection and the standard of clinical common diagnosis;
2. Aged 18-65 years old;
3. Signing the general informed consent form.

Inclusion criteria: 1. Conforming to the diagnostic criteria for the above close contacts;
2. Aged between 18 and 65 years old;
3. Signing the generalized informed consent form. (The intervention objects is incorporated in the clinical study as desired).

Inclusion criteria: 1. Aged 18-75 years old;
2. Patients with novel coronavirus infection pneumonia were confirmed by RT-PCR and clinical symptoms. The diagnostic criteria refer to "Pneumonitis Diagnosis and Treatment Scheme for Novel Coronavirus Infection (Trial Version 5)";
3. Patients with newly diagnosed respiratory system discomfort who have been hospitalized (the diagnosis time of respiratory system discomfort shall not exceed 7 days);
4. Women and

Inclusion criteria: 1. Patients with confirmed new coronavirus-infected pneumonia;
2. Above 18 years old (inclusive);
3. Voluntarily sign written informed consent.

Inclusion Criteria:

The participants were diagnosed as COVID-19 pneumonia, according to the notice on printing and distributing the diagnosis and treatment plan of pneumonia with new coronavirus infection (trial version 4 or update version) made by National Health Commission of the People's Republic of

Inclusion criteria: 1. Diagnosed as novel coronavirus pneumonia (NCP) Patient:
1) Basis of diagnostic criteria: "Notice on Printing and Distributing Pneumonia Diagnosis and Treatment Plan for New Coronavirus Infection (Trial Implementation Fourth Edition)" (National Health Office Medical Letter C2020377)
2019-nCoV diagnosis of pneumonia:
(1) Epidemiological history: A. Travel history or residence history in Wuhan area or other areas with continuous local case

Inclusion criteria: 1. Age over 18 years old;
2. Real-time fluorescent RT-PCR for respiratory or blood specimens to detect novel coronavirus nucleic acid positive;
3. The sequence of virus genes in respiratory or blood samples was highly homologous with the known novel coronavirus;
4. A common or severe type of new type of coronavirus pneumonia was diagnosed. The common patients diagnosed with novel coronavirus pneumonia that have fever, respiratory

Inclusion criteria: 1. Adults (defined as age >= 18 years);
2. Patients with new type of coronavirus infection confirmed by PCR / serum antibodies;
3. The time interval between symptom onset and random enrollment is within 10 days.The onset of symptoms is mainly based on fever. If there is no fever, cough or other related symptoms can be used;
4. Imaging confirmed pneumonia;
5. In the state of no oxygen at rest, the patient's blood oxygen saturation SPO2 <=

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Inclusion criteria: 1. Eligible patients were adults ((aged 18-80 years);
2. Were identified as laboratory-confirmed 2019-nCoV infection;
3. Those who need intubation in ICU, and with no severe hypoxemia.

Inclusion criteria: Patients with novel coronavirus infection who agreed to participate in this trial and signed the informed consent form. The diagnose criteria was according to Diagnostic and treatment protocol of COVID-19 pneumonia published by Chinese General Office of National Health Committee(6th and further edition).

Inclusion criteria: (All the following criteria are met before being selected):
1. Aged >= 18 years;
2. Meet all the following criteria (refer to confirmed cases in the 5th edition of the diagnosis and treatment plan):
(1) Epidemiological history;
(2) Clinical manifestations (according to any of the following 2): fever; normal or decreased white blood cell count or reduced lymphocyte count in the early stages of onset; multiple small patchy shadows and interstitial changes in early chest

Inclusion criteria: 1. Aged >=18 to 70 years;
2. Diagnosed 2019-nCoV infected patients by viral nucleic acid,or although the test was negative and even not be detected, the reseachers judged it to be a novel coronavirus pneumonia;
3. Understand and agree to participate in this study and sign informed consent;
4. According to the guideline of clinical classification of novel coronavirus pneumonia (2019-nCoV) diagnosis and treatment plan (trial version fifth), the general

Inclusion criteria: Conforming to the diagnostic standard of COVID-19; Participants in the first and second trials need to meet the diagnostic criteria of traditional Chinese medicine for epidemic diseases; Participants in the third trial need to meet the diagnostic criteria of deficiency of qi in lung and spleen of epidemic diseases.

Inclusion criteria: 1. Patients who were confirmed diagnosis of 2019-ncov virus pneumonia, according to the "pneumonia diagnosis and treatment program for novel coronavirus infection (trial version 5)",including clinical symptoms, pulmonary CT manifestations and positive nucleic acid test;
2. Inpatients 18 years old (including 18), regardless of gender;
3. Willing to participate in this clinical trial and sign the informed consent

Inclusion criteria: 1. 18 years old <= age <= 60 years old at the time of screening;
2. Pharyngeal swabs, alveolar lavage fluid, or sputum specimens during screening showed that 2019-nCoV virus nucleic acid test results were positive for at least two of nCov-EP, nCov-NP, and nCovORF1ab, in line with the release of a new coronavirus infection by the National Health and Medical Commission Diagnostic criteria for confirmed cases in the pneumonia diagnosis and treatment plan (trial version

Inclusion criteria: 1. The patient signed an informed consent form to participate in the study of convalescent plasma therapy;
2.The age of the patient is 18 or more than18 years old;
3. COVID-19 patients diagnosed by PCR;
4. Nucleic acid positive within 72 hours before blood transfusion;
5. Pneumonia confirmed by imaging;
6. The clinical symptoms reach the standard of severe or critical.
Severe patients meet any of the following:
a) respiratory

Inclusion criteria: 1. Aged 18- 80 years old;
2. Pneumonia patients with new coronavirus infection were confirmed to be positive by RT-PCR and clinical manifestations. The diagnosis standard refers to the diagnosis and treatment plan for pneumonia with new coronavirus infection (trial version 5);
3. Patients with newly diagnosed respiratory system discomfort who have been hospitalized (the diagnosis time of respiratory system discomfort shall not exceed 7 days);
4

Inclusion criteria: 1. Informed consent offered;
2. Confirmed infection with 2019-nCoV by RT-PCR;
3. Compliance with the intervention of study.

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Inclusion criteria: Subjects who meet the following conditions can become subjects of this study:
1. Sign written informed consent. Adolescents or patients who cannot sign informed consent must obtain informed consent from the guardian;
2. Chinese who are >= 18 years of age;
3. It complies with the diagnostic criteria for NCP diagnosis of common patients, that is, the "Pneumonitis Diagnosis and Treatment Scheme for New Coronavirus Infection (Trial Fifth Edition)"

Inclusion criteria: Subjects who meet the following conditions can become subjects of this study:
1. Sign written informed consent. Adolescents or patients who cannot sign informed consent must obtain informed consent from the guardian;
2. Chinese aged >=18 years;
3. It complies with the NCP Critical and Critical Diagnostic Standards, namely "Pneumonitis Diagnosis and Treatment Scheme for Novel Coronavirus Infection (Trial Version 5), Combined with the

Inclusion criteria: (1) Comply with the diagnostic criteria for general type COVID-19 in the "Diagnosis and Treatment Program for COVID-19"(trial version 5th).
(2) Agree to participate in the study and the patient or legal guardian signs the informed consent form.

Inclusion criteria: 1. The patients who were diagnosed with the common type of NCP (including severe risk factors) and severe cases of new coronavirus pneumonia;
2. Aged 18 to 85 years;
3. IL-6 elevated (using Elisa method, using the same company kit);
4. Patients or authorized family members volunteered to participate in this study and signed informed consent.
Definition of Novel Coronavirus Pneumonia (NCP) clinical cases:
1. Regular patients with

Inclusion criteria: 1. Aged 18-75 years male or female;
2. Body weight 40-100 kg and BMI >= 18kg/m2;
3. Been Confirmed with commom 2019-nCoV pneumonia inpatient;
4. With fever, respiratory tract and other symptoms, imaging shows pneumonia;
5. 2019-nCoV nucleic acid test was positive by pathogenic detection within 72 hours;
6. Have undergone surgical sterilization or take effective contraception during the trial;
7. Written the informed consent

Inclusion criteria: Severe patients (according to any of the following): respiratory distress (RR> 30 beats / min); at rest, means oxygen saturation <= 93%; arterial oxygen pressure (PaO2) / oxygen concentration (FiO2) < = 300 mmHg

Inclusion criteria: 1. Patients with confirmed new coronavirus-infected pneumonia;
2. Above 18 years old (inclusive);
3. Voluntarily sign written informed consent.

Inclusion criteria: 1. Hospitalized patients with novel coronavirus pneumonia confirmed by pathogenic detection
2. Meet any one of the criteria for severe type:
1) Respiratory distress: RP >= 30/min;
2) At rest, the oxygen saturation <= 93%;
3) Arterial partial pressure of oxygen (PaO2)/Fraction of inspiration (FiO2)(Oxygenation Index, P/F) <= 300mmHg.
3. Aged 18 to 75 years;
4. Signed informed consent

Inclusion criteria: 1) Aged >= 18 years;
2) In line with the diagnostic criteria of "new coronavirus pneumonia diagnosis program", the patient was diagnosed as pneumonia infected by the new coronavirus;
3) Severe pneumonia index (PSI) class for III ~ IV;
4) The course of symptoms should be less than or equal to 8 days;
5) Volunteers and comply with the hospital regulations, can cooperate with the completion of the prescribed inspection;
6) Voluntary

Inclusion criteria: 1) Aged >= 18 years;
2) In line with the diagnostic criteria of "new coronavirus pneumonia diagnosis program", the patient was diagnosed as pneumonia infected by the new coronavirus;
3) Severe pneumonia index (PSI) class for I ~ II;
4) Hospitalized for:
Fever: axillary temperature >= 36.7 degree C, oral temperature >= 38.0 degree C, Anal temperature >= 38.6 degree C;
respiratory rate > 24 times / min or

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Inclusion criteria: 1. Meet the diagnostic criteria of the "New Coronavirus Pneumonia Diagnosis and Treatment Scheme", patients with pneumonia confirmed by new coronavirus infection;
2. Pneumonia Severity Index (PSI) grade is I ~ II;
3. Aged 18-80 years;
4. Those who have signed the informed consent voluntarily.

Inclusion criteria: 1. General/severe/critical patients who meet the diagnostic criteria of new coronavirus pneumonia;
2. Patients whose viral nucleic acid has turned negative after treatment;
3. Patients who are characterized by respiratory symptoms such as cough, shortness of breath, chest tightness or corresponding chest imaging features;
4. Patients who meet TCM syndrome;
5. Patients who voluntarily participate in the study and sign informed

Inclusion criteria: (1) Comply with the standards of diagnosis and classification of western medicine, and the standards of syndromes of traditional Chinese medicine.
(2) Aged 18-80 years.
(3) Those who voluntarily participated in this research and signed written informed consent.

Inclusion Criteria:

1. Male or female, aged at 18 years-65 years old
2. Pneumonia that is judged by chest radiograph or computed tomography
3. Laboratory confirmation of NCP infection by reverse transcription polymerase chain reaction(RT-

Inclusion criteria: 1. 18 years old <= age <= 60 years old when screening, not limited to men and women;
2. Who are in close contact with suspected cases, confirmed cases, and positive test of new coronavirus nucleic acid, and those who have not taken effective protection, that is, close contacts;
3. Study subjects are willing to be randomly assigned to any designated treatment group;
4. Must agree to not participate in another study of study drugs until day 24 of

Inclusion criteria: 1. 2019 adult patients with new-type coronavirus pneumonia diagnosed with 2019-nCoV infection by PCR according to the "Pneumonitis Diagnosis and Treatment Protocol for New-type Coronavirus Infection (Trial Version 5)"
2. The absolute value of lymphocytes <0.6x10^9/L;
3. Severe respiratory failure does not exceed 48 hours and requires ICU treatment. Among them, severe respiratory failure is defined as PaO2 / FiO2 <200 mmHg and requires positive

Inclusion criteria: (1) Those who meet the diagnosis of new coronavirus pneumonia and have clinical classification of low and medium;
(2) Those who have been diagnosed with TCM as epidemic-closed lungs;
(3) Inpatients aged 18 to 75 years, regardless of gender;
(4) Voluntarily receive treatment with the drug and sign informed consent. The informed consent process complies with relevant GCP regulations

Inclusion criteria: 1. Age is not limited;
2. Clinical diagnosis is in accordance with the "Notice on issuing a new type of coronavirus pneumonia diagnosis and treatment program (Fifth edition)" on the diagnosis of a new type of coronavirus infection;
3. potable decoction;
4. no honeysuckle allergy;
5. child-bearing age female subjects pregnancy test negative person;
6. child-bearing age female subjects pregnancy test positive person needs targeted

Inclusion criteria: 1. Patients who meet the requirements of the New Coronavirus Infected Pneumonia Diagnosis and Treatment Scheme (revision of Trial Version 5);
2. Aged 18-75 years male or female;
3. Serious patient meet one of the following:
(1) respiratory distress wiht respiratory rate >=30 breaths/min;
(2) oxygen saturation<=93% on quiet status;
(3) Arterial partial pressure of oxygen(PaO2)/oxygen concentration<=300mmHg;
4. critically ill patients

Inclusion Criteria:

CT image is characteristic of viral pneumonia;
2019-ncov infection (positive nucleic acid test) is confirmed by pathogenic test;
In compliance with the 2019-nCoV infection severe pneumonia diagnosis standard (according to the

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Inclusion Criteria:

Aged between 18 and 80 years (Including 18and 80years, male or female).
One of them:

Inclusion criteria: Subjects who meet the all of the following criterion to be elegiable for this study:
1. Meet the requirements of the "New Coronavirus Infected Pneumonia Diagnosis and Treatment Scheme (revision of Trial Version 5)";
2. Aged 18 to 65 years male or female;
3. The interval from the onset of symptoms to enrollment is within 7days: onset of symptoms is defined as onset of at least one systematic symtom(such as fever(temperature>=37.3 degree C) or

Inclusion criteria: Participants with 18 to 75 years of age who have been diagnosed with NCP by nucleic acid testing and meet the clinical classification of severe NCP referring to the clinical classification standard in the General Office of the National Health and Health Commission-Novel Coronavirus Pneumonia Diagnosis and Treatment Program (Trial Version 4).

Inclusion criteria: 1. Severe patients diagnosed with novel coronavirus pneumonia (COVID-19);
2. Aged 35 years old to 74 years old;
3. Patients or authorized family members volunteered to participate in this study and signed informed consent.
Note: Case definition: Severe NCP: Refer to the "New Coronavirus Pneumonia Diagnosis and Treatment Plan (Fifth Edition)" formulated by the National Health Commission.

Inclusion criteria: (1) aged >=18 years old;
(2) real-time fluorescence rt-pcr of respiratory or blood samples presents the positive nucleic acid of COVID-19, or the viral gene sequencing of respiratory or blood samples was highly homologous with the known COVID-19.
(3) patients diagnosed with novel coronavirus meet the diagnostic criteria of the latest clinical guidelines for novel coronavirus issued by the world health organization (WHO) on January 28, 2020 and the

Inclusion criteria: 1) common type pneumonia patients with confirmed new coronavirus infection;
2) aged 18 to 75 years old;
3) voluntary signing of written informed consent.

Inclusion criteria: 1. Subjects have signed informed consent; agree not to participate in other clinical studies within 30 days from the first administration of the research drug to the last administration.
2. Aged >= 18 and <= 75 years.
3. Met the diagnostic criteria for COVID-19 (fifth edition, China).
4. SOFA score: 2 to 13 points.
COVID-19 clinical stratifications:
1. Mild (n = 200): The clinical symptoms are mild or asymptomatic, and no pneumonia is seen on imaging; but

Inclusion criteria: 1. Aged 18 years old and above;
2. Meet the novel coronavirus pneumonia (COVID-19) diagnostic criteria. The upper and lower respiratory tract RT-PCR confirmed that 2019-nCoV nucleic acid positive, chest CT imaging examination could be used in conjunction;
3. SaO2/SpO2=94% under indoor air, or PaO2/FiO2<300mgHg (for severe type patients);
4. Sign informed consent;
5. Do not participate in the clinical study of other study drugs within 28 days

Inclusion criteria: 1. patients with novel coronavirus pneumonia confirmed by pathogenic detection;
2. Fever, respiratory tract and other symptoms, the change of pneumonia can be seen in imaging;
3. Aged 18 to 80 years;
4. Signed informed consent.

Inclusion Criteria:

age more than 18 years old
accord with the clinical diagnosis and/ or etiological diagnosis diagnostic criteria of Novel coronavirus pneumonia (COVID-19)

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Inclusion criteria (I) Discharged patients fulfilling the diagnostic criteria of COVID-19 (China Diagnosis and Treatment Protocol for COVID-19 (Trial Version 7); (II) COVID-19 RNA in respiratory specimens or blood specimens of patients is negative (>2 times), assayed by real-time fluorescent polymerase chain reaction test (RT-PCR); (III) Pulmonary CT scans within 7 days showed that there were still unabsorbed inflammation or pulmonary fibrosis in the lungs; (IV) Age 18-70

Inclusion criteria: 1. Age 18 to 75 Yearsmale or female subjects
2. Diagnosed with COVID-19,according to "New Coronavirus Infected Pneumonia Diagnosis and Treatment Scheme (Pilot Sixth Edition);
3. Sever COVID-19, except shock and ICU care with organ failure;
4. Antimalarial drugs such as chloroquine and hydroxychloroquine sulfate were not used within 3 months ;
5. Agreed not to participate in other clinical trials in the period of 30 days from first

Inclusion criteria: 1. Aged >=18 years at time of signing Informed Consent Form;
2. Patients with confirmed cases meeting the following criteria
(1) Epidemiology history;
(2) Clinical manifestations (epidemiological history and in accordance with any of the following 2 items; no clear epidemiology history, conforms to 3 items in clinical manifestations):
Fever and / or respiratory symptoms;
In the early stage of the disease, the total leukocyte count was normal or

Inclusion criteria: Patients aged 18 or older, and meet the diagnostic criteria of Diagnosis and Treatment Scheme of Novel Coronavirus Infected Pneumonia published by the National Health Commission.
Criteria for diagnosis (meet all the following criteria):
1. With epidemiological history;
2. Clinical manifestations (meet any 2 of the following): fever, normal or decreased white blood cell count or lymphopenia in the early stage of onset, and Chest radiology in the early

Inclusion criteria: 1. Meet the diagnostic criteria for suspected and confirmed cases (including mild, general, and severe) in the Diagnosis and Treatment of Pneumonia of New Coronavirus Pneumonia (Trial Version 5);
2. Meet the TCM diagnostic criteria for each stage of the novel coronavirus pneumonia;
3. Aged 18-75 years-old men or women;
4. Patients who volunteer to participate in this study and sign informed consent

Inclusion criteria: 1. Meet the diagnostic criteria forconfirmed cases in the Diagnosis and Treatment of Pneumonia of New Coronavirus Pneumonia (Trial Version 5);
2. The acute stage is complicated by sepsis, respiratory failure, ARDS and so on. The patients are with multiple organ damage when improved and tended to recover after organ support and other treatments;
3. Meet the TCM diagnostic criteria for the recovery period;
4. Aged 18-75 years-old men or

Inclusion criteria: 1. Diagnosed Novel coronavirus pneumonia patients or clinically diagnosed NCP patients in Hubei Province;
2. Aged 18 to 65 years;
3. The time interval between symptom onset and randomization was within 7 days (the onset of symptom was mainly determined by the time of fever);
4. Sign the written informed consent before the study.

Inclusion criteria: cured patients of 2019 novel coronavirus.

(1) Aged 18-65 years;
(2) Virus nucleic acid test is positive;
(3) Lung CT showed multiple patchy, lumpy ground glass opacity(GGO), consolidation or other typical manifestations in both lungs .

Inclusion criteria: 1. Patients or his/her legal representatives agreed to participate the study and signed the informed consent.
2. Those diagnosed with mild or moderate novel coronavirus pneumonia (NCP) or those with severe NCP but can take medicines orally or via stomach tube. The confirmed patients were defined as those NCP suspects with one of the following pathogenic evidences:
1) The novel coronavirus in respiratory or blood specimens were confirmed by real

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Inclusion criteria: 1. Aged 18 to 60 years old;
2. Inpatients who meet the diagnostic criteria for mild, general and neonatal pneumonia in western medicine, and early and recovery stages of traditional Chinese medicine;
3. Understand and agree to participate in this study and sign the informed consent.

Inclusion criteria: 1) Male and female, 18-70 years old;
2) Severe patients (in any case: respiratory distress, RR >= 30 times / min; in resting state, oxygen saturation <= 93%; PaO2 / FiO2 <= 300MMHG);
3) Within 12 days after illness onset;
4) Agree to sign the informed consent;
5) Able to ingest, absorb and tolerate oral medication;
6) Agree to collect clinical samples;
7) Consent to contraceptive measures for sex within 7 days of the last oral

Inclusion criteria: 1. Aged >=60 years;
2. Patients confirmed with novel coronavirus pneumonia. The Diagnostic criteria refer to "Pneumonitis Diagnosis and Treatment Scheme for Novel Coronavirus Infection (Trial Version 5)";
3. Those without broad-spectrum antibacterial, antifungal, and antiviral treatment;
4. Imaging revealed pneumonia in patients with a diagnosis of pathogen: influenza, CAP/HAP bacterial infection, and fungal infection;
5. Participants signed

Inclusion criteria: 1. Male or female, 18-95 years old;
2. 2019 new coronavirus nucleic acid test positive;
3. Diagnosed as moderate to severe cases of new coronavirus pneumonia in 2019, which meets the following clinical manifestations:
4. With fever and respiratory symptoms, imaging shows pneumonia;
5. A severe case diagnosed with 2019 new coronavirus pneumonia, that is, any of the following:
(1) Increased breathing rate (>=30 beats / min), difficulty breathing

Inclusion criteria: 1. Patients withmild and moderate novel coronavirus pneumonia (COVID-19) diagnosed refers to the diagnosis and treatment plan for pneumonia with new coronavirus infection (trial version 5);
2. Patient cough score > 1;
3. Aged 18- 80 years old;
4. The subjects (including male subjects) had no pregnancy, sperm donor, egg donor for nearly half a year, and were willing to take effective contraception from the first dose until three months after the last

Inclusion criteria: 1. The age at the time of signing the informed consent is >= 18 years old and <= 75 years old, both male and female;
2. The change of typical viral pneumonia in lung was detected by RT-PCR test of novel coronavirus nucleic acids or lung CT;
3. SaO2 / SpO2 <= 94% or pa02 / fi02 ratio < 300mghg at room temperature at admission;
4. Onset <=12 days;
5. Willing to participate in the trial and randomly assigned to any treatment group;
6

Inclusion criteria: 1. Aged>= 18, male or female;
2. Fever, axillary temperature > 37.3 degrees;
3. Consistent with the clinical diagnosis of COVID-19 mild, normal, severe case;
4. Patients who sign informed consent form freely and voluntarily.

Inclusion criteria: (1) Meet NCP critical diagnosis criteria;
(2) Volunteer to participate in this study, cooperate with examination and treatment;
(3) The selected patients were basically stable, conscious and cooperative;
(4) No serious mental disorder;
(6) Aged between 20-80 years.

Inclusion criteria: (1) Adult aged >=18 years and agrees to sign the informed consent;
(2) Laboratory-confirmed 2019-nCoV infection by real-time RT-PCR;
(3) Chest imaging (CT) confirmed lung damage: multiple small plaques and stromal changes in the lungs, which were obvious in the outer lung, or multiple ground-glass shadows and infiltration shadows in both lungs, the changes may not be present in mild patients;
(4) Hospitalized patients with fever (axillary

Inclusion criteria: 1. The patients who were diagnosed with the common type of NCP (including severe risk factors) and severe cases of new coronavirus pneumonia;
2. Aged 18 to 85 years;
3. IL-1ÃŸ elevated (using Elisa method, using the same company kit);
4. Patients or authorized family members volunteered to participate in this study and signed informed consent;
Definition of Novel Coronavirus Pneumonia (NCP) clinical cases:
1. Regular patients with

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Inclusion criteria: (1) Patients with mild who are in compliance with the "Pneumonitis Diagnosis and Treatment Program for COVID-19(Trial Fifth Edition)" (released on 2020.2.4);
(2) Aged more than 18 years old;
(3) Informed consent of the patient.

Inclusion criteria: (1) In line with ethics, sign a written informed consent form;
(2) Patients with positive detection of new coronavirus nucleic acid;
(3) Chest CT confirmed pulmonary lesions;
(4) Hospitalized patients with fever or respiratory symptoms;
(5) Patients who did not participate in other clinical studies within three months.

Inclusion criteria: 1. Patients aged 15-80 years,diagnosed with mild, common, and severe novel coronavirus pneumonia in accordance with the diagnostic criteria for the diagnosis and treatment of novel coronavirus pneumonia issued by the National Health Commission (trial version 5);
2. White blood cell Count < 15.0 x 10^9/L and Lymphocyte Count < 0.8 x10^9/L.

Inclusion criteria: 1. Aged 18 to 70 years old, inpatients, male or female;
2. Patients with severe novel coronavirus infection: According to the "Pneumonitis Diagnosis and Treatment Guideline for the Novel Coronavirus Infection (Trial Version 5)", clinically diagnosed cases (suspected cases with pneumonia imaging features) or suspected cases.
Severe patients must also meet any of the following:
1) Respiratory distress, RR>=30 times / minute
2) In the resting state, the

Inclusion criteria: Patients aged >=18 years with novel coronavirus pneumonia who agreed to participate in this trial and signed the informed consent.

Inclusion Criteria:

Age ≥18 years at time of signing Informed Consent Form.
Laboratory (RT-PCR) confirmed infection with CoVID-19.
Hospitalised with Fever(>36.7°C, axilla or oral temperature > 38.0 °C or >38.6°C tympanic or rectal)

Inclusion criteria: 1. Voluntarily sign the Consent Form;
2. Male or female aged 18 and over;
3. According to the "Diagnosis and Treatment Scheme for Pneumonia of Novel coronavirus pneumonia Infection-Trial Fifth Edition", inpatient diagnosed with Novel coronavirus pneumonia diagnosed and clinical classification of ordinary type: Inpatients with fever (underarm temperature >= 37.0 degree C), respiratory tract, etc. Imaging shows pneumonia

Inclusion criteria: pneumoniaits were diagnosed by chest CT findings and positive nucleic acid tests

Inclusion criteria: (1) Age 18 to 80 years old (including threshold), sex unlimited, inpatient
(2) Respiratory specimen were positive for COVID-19 nucleic acid by RT-PCR;
(3) Clinical classification is mild or moderate type (with symptoms of fever, respiratory tract, imaging findings of pneumonia);
(4) Hospitalization for the following reasons:Fever (axillary temperature >= 36.7 degrees C, or mouth temperature >= 38.0 degrees C, or anal or ear temperature >= 38.6 degrees C)

Inclusion criteria: (1) RT-PCR detection of the respiratory or blood samples shows positive nucleic acid of the novel coronavirus;
(2) Gene sequence of the virus in respiratory or blood samples are highly homologous to the known coronaviruses;
(3) The CT images of chest shows radiographic features of pneumonia;
(4) Patients with severe or critical conditions;
(5) Aged >=18, male or female;
(6) The subjects should volunteer to join the study, sign the informed

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Inclusion criteria: 1. Meet the requirements of the "New Coronavirus Infected Pneumonia Diagnosis and Treatment Scheme (revision of Trial Version 5)";
2. Aged 18 to 65 years male or female;
3. The interval from the onset of symptoms to enrollment is within 7days: onset of symptoms is defined as onset of at least one systematic symtom(such as fever(temperature>=37.3 degree C) or shivering headache hypodynamia muscle pain or joint pain) or respiratory symptom(such as

Inclusion criteria: 1. Aged 18 to 80 years (inclusive);
2. People who had ineffective protection, i.e.:
1) living, studying, working together with, or having close contacts with confirmed cases and positive testers, such as working at close distances or sharing the same classroom or living in the same house;
2) had close contacts with medical personnel, family members or people who have similar close contact during the diagnosis, treatment, and visits, such as visiting or staying in a

Inclusion criteria: (1) Patients in accordance with the diagnostic criteria of "Quick reference guide for diagnosis and treatment of novel coronavirus pneumonia(2019-nCoV)(Standard Edition)" and "Diagnostic and treatment protocols of novel coronavirus pneumonia(Trial version 5)" issued by National Health Commission;
(2) Patients in accordance with the TCM syndrome differentiation standard;
(3) Aged 18 to 85 years;
(4) Before the trial begins, patients or their legal

Inclusion criteria: 1. Confirmed and suspected cases of pneumonia from novel coronavirus infection;
2. Meet the diagnostic criteria for community - acquired pneumonia;
3. Clinical classification is "heavy" or "critical";
4. Aged 18 to 75 years;
5. Obtain informed consent.

Inclusion Criteria:

Laboratory diagnosis:

Respiratory specimen is positive for SARS-CoV-2 nucleic acid by RT-PCR; OR

Inclusion criteria: 1. Aged 18-75 years old;
2. RT-PCR positive for 2019-nCoV or confirmed lung involvement with chest imaging;
3. The clinical symptoms are slight, and there is no obvious manifestation of pneumonia (mild disease) or clinical symptoms such as fever and respiratory tract in imaging. The manifestation of pneumonia (common type) can be seen in imaging;
4. <=7 days since onset of the disease;
5. Willingness of study participant to accept

Inclusion criteria: Suspected cases of neo-coronary pneumonia (outside Hubei province), clinically diagnosed cases (in Hubei province), and light / general confirmed cases, severe/critical confirmed cases, aged 18-70 years, each center enrolls cases in a competitive manner.
The diagnostic criteria for the new coronavirus pneumonia conform to the diagnostic criteria issued by the National Health and Medical Commission for the diagnosis and treatment of pneumonia in the new

Inclusion criteria: 1. aged 18-70 years male or female;
2. 2019 new coronavirus nucleic acid test positive;
3. meet the national diagnosis criteria;
4. subjects onset within 8 days (severe cases can be delayed to 14 days);
5. subjects or agency personnel agree to sign the informed consent;
6. subjects who can take oral medication or who receive oral medication;
7. subjects agree to collect clinical samples;
8. female subjects of child-bearing age are not

Inclusion criteria: Patients with mild/common novel coronavirus pneumonia who agreed to participate in this trial and signed the informed consent form

Inclusion criteria: 1. Age between 18 and 75 years old (including threshold), regardless of gender;
2. The patient under isolation observation is diagnosed with mild or common type of 2019-nCoV acute respiratory disease and can cooperate to complete corresponding psychological intervention;
3. The total score of Hamilton depression scale (HAMD17) and Hamilton anxiety scale (HAMA14) at baseline is both at least 7;
4. The patient has sufficient compliance to complete

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Inclusion criteria: 1) age 18-80 years old, regardless of gender: 2) the diagnosis criteria of severe nCoV pneumonia in line with the 6th edition of China's new coronavirus pneumonia diagnosis scheme: a.B. Real-time fluorescence rt-pcr of respiratory tract specimens or blood samples was used to detect positive nCoV nucleic acid;3) critical cases of nCoV pneumonia: meet one of the following criteria: a. respiratory failure and mechanical ventilation required;B. shock;C. Combined

Inclusion criteria: Subjects who meet the following conditions can become subjects of this study:
1. Will sign written informed consent. Adolescents or patients who cannot sign informed consent must obtain informed consent from the guardian;
2. Chinese aged >=18 years;
3. In accordance with NCP criteria for severe and critical illness, namely "Pneumonitis Diagnosis and Treatment Scheme for Novel Coronavirus Infection (Trial Version 6)". Combined with the following

Inclusion criteria: 1. Aged 18-80 years;
2. Within 1 week of onset;
3. Meet the diagnostic criteria of common, severe and critical NCP, according to "Pneumonitis Diagnosis and Treatment Scheme for Novel Coronavirus Pneumonia (Trial Version 6)".
4. Sign informed consent.

Inclusion criteria: Patients are eligible for admission if they meet all of the following criteria within 5-7 days of their first ICU admission.
1) Aged 18-75 years;
2) The patient meets the ARDS diagnostic standard (Berlin standard) and needs intubation ventilator-assisted or ECMO-assisted treatment;
3) The primary disease caused by 2019-nCoV virus infection;
4) Imaging data suggest bilateral lung lesions;
5) Voluntarily participate in clinical research; fully understand

Inclusion criteria: 1. Aged 18-70 years old;
2. Meet the diagnostic criteria for confirmed cases in the diagnosis and treatment plan of new coronavirus-infected pneumonia (trial version 5); and the clinical symptoms before admission or screening must meet the following conditions:
Fever (armpit temperature >=37.3 degree C, or mouth temperature >=38.0 degree C, or anal or ear temperature >=38.6 degree C) <=7 days; and breathing rate> 24 beats / min or cough (at least one

Inclusion criteria: (1) All patients were diagnosed with a new type of coronavirus pneumonia;
(2) Clear mind, cooperation and communication;
(3) Men and women aged 18 and 80 years;
(4) informed consent has been signed and is willing to be assessed.

Inclusion criteria: 1. Aged>=18 years old;
2. Diagnosed as severe and critical patients with novel coronavirus pneumonia;
3. Being treated in the ICU.

Inclusion criteria: Subjects who met all of the following conditions entered the trial:
1. Male or female patients aged 16 to 75 years;
2. willing to sign informed consent;
3. Based on the criteria for suspected cases of COVID-19 infection, sputum, throat swabs, lower respiratory tract secretions and other specimens were tested for positive COVID-19 nucleic acid by real-time fluorescent RT-PCR; or viral gene sequencing was performed with known COVID-19 highly

Inclusion criteria: inclusion of case criteria (pneumonia diagnosis and treatment protocol for COVID-19 (trial 6th edition))
1. Common type patient: with fever, respiratory tract and other symptoms, and imaging findings of pneumonia.
2. Severe patients: patients who meet any of the following criteria
(1) respiratory distress, RR>=30 times/min;
(2) in resting state, oxygen saturation is less than 93%;
(3) partial arterial oxygen pressure (PaO2)/oxygen absorption concentration

Inclusion criteria: (1) Those who meet the diagnosis of new coronavirus pneumonia and have clinical classification of low and medium;
(2) Inpatients aged 18 to 75 years;
(3) Voluntarily receive treatment with the drug and sign informed consent. The informed consent process complies with relevant GCP regulations.

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Inclusion criteria: 1. Aged 18-60 years old;
2. Patients with clinical symptoms, signs, auxiliary detection combined with 1 or more nucleic acid test to confirm the diagnosis of new coronavirus pneumonia;
3. Voluntarily sign the informed consent to comply with the requirements of the program.

Inclusion criteria: 1) The confirmed patient (or legal guardian) agrees to participate in the study and signs the informed consent form;
2) aged 18 to 65 years;
3) Real-time fluorescent RT-PCR of respiratory specimens or blood specimens to detect patients with new type of coronavirus positive;
4) Patients diagnosed as severe and critically ill and with rapid disease progression according to the "Diagnosis and Treatment Program for Pneumonia of New Coronavirus Infection

Inclusion criteria: 1. Sign the written informed consent;
2. Aged 18-80 years old;
3. Meet the diagnostic criteria for novel coronavirus infection(trial version sixth), confirmed by combining with the epidemiological history, clinical manifestations and etiology examination;
4. Course of disease within 14 days after onset.

Inclusion criteria: (1) Comply with the diagnostic criteria for COVID-19 in the "Diagnosis and Treatment Program for pneumonia caused by novel coronavirus infection (Trial Version 5)", meet the the criteria of severe and critical type;
(2) Agree to participate in the study and the patient or legal guardian signs the informed consent form.

Inclusion Criteria:

Male or female, aged at 18 years (including) -75 years old
Hospitalized
Laboratory confirmation of SARS-CoV-2 infection by reverse-transcription polymerase chain

Inclusion Criteria:

Male or female, 18 years old ≤ age ≤ 75years old;
CT image is characteristic of 2019 novel coronavirus pneumonia;
Laboratory confirmation of 2019-nCoV infection by reverse-transcription polymerase chain reaction

Inclusion criteria: 1. Patients discharged from severe or critical new coronavirus pneumonia;
2. Adults over 18 years of age who signed the informed consent;
3. Study participants are willing to accept random assignment to any designated treatment group;
4. Prior to the completion of this study, participants must agree not to participate in another study.

Inclusion criteria: 1) The confirmed patient (or legal guardian) agrees to participate in the study and signs the informed consent form;
2) aged 18 to 65 years;
3) The time interval between the onset of symptoms and random enrollment is within 8 days (the onset of symptoms is mainly based on the occurrence of fever);
4) Real-time fluorescent RT-PCR of respiratory specimens or blood specimens to detect patients with 2019-nCoV nucleic acid positive;
5) Patients were diagnosed

Inclusion criteria: (1) Aged >=18 years;
(2) Novel coronavirus pneumonia patients diagnosed by pathogenic testing;
(3) The patient himself participated in the study voluntarily, agreed and signed the informed consent.

Inclusion criteria: 1. Female patients aged >=18 years who were of childbearing years were excluded from pregnancy and agreed to use non-drug contraceptive measures during the study period;
2. SARS-CoV-2 nucleic acid test positive;
3. Lung CT examination showed evaluable lesions;
4. Meet the diagnostic criteria for severe or critical illness in the diagnosis and treatment program for new coronavirus pneumonia (trial version 6);
5. Subjects voluntarily

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Inclusion criteria: 1. Aged 18 to 80 years;
2. Patients with novel coronavirus pneumonia who were diagnosed as general type or severe type according to the western medicine classification general type, severe type and Chinese medicine classification criteria;
3. Understand and agree to participate in the study and sign the informed consent.

Inclusion criteria: (1) Aged 18 years or older;
(2) Voluntarily signed informed consent;
(3) Hospitalized patients diagnosed as COVID-19.

Inclusion criteria: 1. Patients with confirmed new coronavirus-infected pneumonia, clinical classification is ordinary or heavy;
2. TCM syndrome differentiation is wind-heat syndrome;
3. Aged between 18-75 years old, regardless of gender;
4. Voluntarily sign written informed consent.

Inclusion criteria: 1. Aged 18 to 75 years old;
2. Pneumonia patients with new coronavirus infection were confirmed to be positive by RT-PCR and clinical manifestations. The diagnosis standard refers to the current diagnosis and treatment plan for pneumonia with new coronavirus infection;
3. Patients with newly diagnosed respiratory system discomfort who have been hospitalized (the diagnosis time of respiratory system discomfort shall not exceed 7 days);
4

Inclusion criteria: 1. novel coronavirus pneumonia patients with nutritional malnutrition diagnosed by nutritional assessment;
2. enteral nutrition support treatment for at least 5 days;
3. No use of Fat emulsion taboo;

Inclusion criteria: Patients meet the diagnostic criteria "Diagnosis and treatment of novel coronavirus pneumonia (trial version fifth)"for suspected cases outside Hubei Province: combined with the following epidemiological and clinical manifestations: epidemiological history
1. Patients with travel or residence history in Wuhan and surrounding areas or other communities with case reports within 14 days before the onset of the disease;
2. Patients with a novel coronavirus infection

Inclusion criteria: 1. Aged 18 to 85 years old;
2. It conforms to the diagnostic criteria of severe and critical type in "Diagnosis and Treatment Plan for Pneumonia Infected in novel coronavirus (6th Edition)";
3. Sign the informed consent form.

(1) Comply with the diagnostic criteria for general type COVID-19 in the "Diagnosis and Treatment Program for COVID-19"(trial version 6th).
(2) Aged >= 18 years.
(3) Agree to participate in the trial, and the patient, the legal guardian or the person in charge of the medical institution signed the informed consent through paper signature, electronic signature of

Inclusion criteria: 1. First diagnosis of novel coronavirus infection
2. Epidemiological history Travel history or residence history of Wuhan City within two weeks before the onset of illness; or patients who have had fever from Wuhan with respiratory symptoms within 14 days before the onset of illness, or have cluster Onset;
3. Clinical manifestations:
(1) fever;
(2) with the following imaging characteristics of pneumonia: early lesions are limited, showing patchy

Inclusion criteria: Patients undergoing painless gastroscopy in our hospital.

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Inclusion criteria: (1) Comply with the diagnostic criteria for general type COVID-19 in the "Diagnosis and Treatment Program for COVID-19"(trial version 6th);
(2) Patients =18 years of age with tracheal intubation;
(3) Agree to participate in the trial, and the patient, the legal guardian or the person in charge of the medical institution signed the informed consent through paper signature, electronic signature of mobile phone software, or voice authorization.

Inclusion criteria: 1. New-type coronavirus infection is diagnosed as mild or common (diagnosed according to the "Notice on the Issuance of a New Coronavirus-Infected Pneumonia Diagnosis and Treatment Plan (Trial Implementation of the Sixth Edition)") or patients with poor immune function (cured discharged patients but peripheral hemolymph Cell count and classification are abnormal).
2. Aged 18-65 years;
3. Sign informed consent.

Inclusion criteria of Cohort 1 #4 is adapted from the Flu guideline in China (2018) and community acquired pneumonia (CAP) guideline in China (2016)

Cohort 1:

Inclusion criteria: 1. Aged 18 to 70 years old;
2. Patients with pneumonia infected by new coronavirus: confirmed cases in compliance with the Diagnosis and Treatment Plan for Pneumonia Infected by New Coronavirus (Sixth Edition);
3. Confirmed cases who can be transfused into anti-SARS-CoV-2 virus inactivated plasma after fully assessing patients condition;
4. Cases who score 3 or 4 according to 6-grade scale;
5. The subjects and/or their legal guardians

Inclusion criteria: 1. The patient signs the written informed consent;
2. Aged 18-80 years;
3. the novel coronavirus novel coronavirus infection diagnostic criteria for pneumonia, namely, the new coronavirus pneumonia diagnosis and treatment plan (trial version sixth). It conforms to any of the following:
1) in case of shortness of breath, RR >= 30 times / min;
2) in case of resting state, oxygen saturation <= 93%;
3) in case of arterial partial pressure of oxygen (PaO2) /

Inclusion criteria: 1. Aged 18 to 75 years men and women;
2. Inpatients with novel coronavirus(SARS-CoV-2) infection confirmed by PCR;
3. Agree and sign the informed consent form.

Inclusion criteria: 1. Aged 18 to 65 years old, both male and female;
2. The eligible subjects (male and female) with fertility must agree to take effective contraceptive measures from the beginning of self-sufficiency with their partners to 6 months after drug withdrawal;
3. Novel coronavirus pneumonia diagnosis novel coronavirus pneumonia novel coronavirus pneumonia diagnosis and treatment plan (latest edition) is confirmed;
4. Drugs without chloroquine and its

Inclusion criteria: 1. Aged 18-80 years;

2. Meet the diagnostic criteria of "New Coronavirus Infected Pneumonia Sixth Edition";

3. Blood pressure 90-150 / 60-100mmHg, respiratory rate 12-20 beats / min, heart rate 60-100 beats / min;

4. Extremity muscle strength examination is more than 5-grade;

Inclusion criteria: 1) confirmed or suspected cases of new coronavirus pneumonia;
2) be able to understand the content of this study, and have the ability of normal communication and understanding;
3) voluntarily participate in this study and obtain written informed consent from the guardian or the patient.

Inclusion criteria: (1) Aged 20-45 years old;
(2) Understand and willing to participate in this clinical trial and provide signed informed consent.

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Inclusion criteria: 1. Eligible patients were adults ((aged 18-80 years);
2. Were identified as laboratory-confirmed COVID-19 infection;
3. Those who need intubation in ICU, and with no severe hypoxemia.

Inclusion criteria: 1) confirmed or suspected cases of new coronavirus pneumonia;
2) be able to understand the content of this study, and have the ability of normal communication and understanding;
3) voluntarily participate in this study and obtain written informed consent from the guardian or the patient.

Inclusion criteria: 1. Real-time fluorescent RT-PCR detection of new coronavirus nucleic acid positive, or viral gene sequencing is highly homologous with known new coronavirus;
2. Confirmed as a novel coronavirus pneumonia common and severe patients, meets the diagnostic criteria of the "novel coronavirus pneumonia diagnosis and treatment plan (trial implementation of the sixth edition)"
3. aged 18 to 65 years;
4. Learn about the significance of the study

Inclusion criteria: (1) Patients aged from 18 to 75 years old;
(2) Mild patients with confirmed COVID-19 according to the diagnostic criteria of "novel coronavirus Infection Diagnosis and Treatment Plan (Trial 6th Edition)" issued by the National Health and Safety Commission;
(3) Patients and their families voluntarily participated in the study and signed an informed consent form.

Inclusion criteria: (1) diagnosed as a mild and common type of the new coronavirus pneumonia, meeting the diagnostic criteria of the new coronavirus pneumonia diagnosis plan (trial version 6), both male and female;
(2) aged 14 to 70 years old;
(3) body temperature > 37.3 degree C;
(4) understand the nature, significance, possible benefits, inconvenience and potential dangers of the test in detail before the test;
(5) understand the study procedure and voluntarily sign the

Inclusion criteria: 1) Confirmed or clinically diagnosed patients with the novel coronavirus disease;
2) Those who have been identified by Chinese medicine as pestilence stagnant in Lung;
3) Inpatients aged 18 to 75 years male or female;
4) Volunteer to receive treatment under their own scheme and sign informed consent.

Inclusion criteria: 1) Confirmed or clinically diagnosed patients with the novel coronavirus disease;
2) Those who have been identified by Chinese medicine as pestilence stagnant in Lung;
3) Inpatients aged 18 to 75 years male or female;
4) Volunteer to receive treatment under their own scheme and sign informed consent.

Inclusion criteria: 1. Aged >=18 years;
2. Clinically diagnosed patients with COVID-19, including: on the basis of meeting the criteria for suspected cases, one of the following etiology evidence:
(1) real-time fluorescent RT-PCR detection of SARS-Cov-2 nucleic acid samples in respiratory or blood samples;
(2) Sequencing of viral genes in respiratory specimens or blood specimens, highly homologous to known SARS-Cov-2;
3. The interval between the onset of

Inclusion criteria: Patients who are more than 18 years are definitely diagnosed with COVID-19. [That is, the diagnosis of 2019-nCoV-infected pneumonia patients was diagnosed according to the diagnostic criteria for novel coronavirus pneumonia diagnosis and treatment program (trial version 5) issued by the National Health and Health Commission on February 5, 2020.

Inclusion criteria: (1) age: 18-75 years old (including threshold), regardless of gender;
(2) compliance with the pneumonia diagnosis and treatment protocol for novel coronavirus infection - the diagnostic criteria for the trial version 6 (the latest version, which is currently updated to version 6).
(3) the sixth version of the reference treatment scheme meets the common type standard;
(4) no participation in traditional health exercise (ba duan jin, tai chi, wu qin xi or other fitness

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Inclusion criteria: (1) The novel coronavirus was detected at the age of 18~70 years when signing the informed consent; (2) the nucleic acid test of the pharyngeal test, sputum, lower respiratory tract secretions, blood and so on (RT-PCR detection of the new coronavirus nucleic acid positive) and the sequencing of the virus gene, which were highly homologous with the known new coronavirus, and confirmed that the 2019-nCoV was detected. Chest X * ray and CT imaging confirmed pneumonia

Inclusion Criteria:

Patients with mild and severe cases who have been diagnosed with new coronavirus pneumonia according to the "Pneumonitis Diagnosis and Treatment -
Plan for New Coronavirus Infection"

Inclusion criteria: 1. Male or female aged 18-80 years old;
2. Patients with confirmed novel coronavirus-infected pneumonia;
3. Diagnosed as moderate cases of COVID-19 with fever and respiratory symptoms and image showing pneumonia;
4. Voluntarily sign written informed consent.

Inclusion criteria: (1) Aged 18 years or older, male or female;
(2) Real-time fluorescent RT-PCR of pharyngeal swab samples was positive for novel coronavirus nucleic acid;
(3) Clinical classification is common or severe;
(4) Signed informed consent.

Inclusion criteria: 1. Laboratory confirmed infection with 2019-nCoV;
2. Clinical classification is mild or common;
3. Lung involvement confirmed with chest imaging;
4. Hospitalised with: Fever: axilla temperature >= 36.7 degrees C, or Oral temperature >= 38.0 degrees C, or rectal temperature >= 38.6 degrees C.And at least one of Respiratory rate > 24/min Or Cough;
5. TCM syndrome differentiation is phlegm-heat obstructing the lung syndrome;
6. Must agree not

Inclusion criteria: 1. Aged 18 to 75 years;
2. Real-time fluorescence RT-PCR detection of nucleic acid positive of novel coronavirus in respiratory tract specimens or blood preparation;
3. Sequencing of virus genes in respiratory tract specimens or blood preparation, highly homologous to known novel coronavirus ;
4. The patients are diagnosed with common type of novel coronavirus pneumonia according to the Pneumonia Diagnosis and Treatment Program for Novel

Inclusion criteria: 1. Adult patients confirmed by 2019-nCov throat swab or other samples of nucleic acid positive;
2. Patients with clear manifestations of pulmonary exudation on chest imaging;
3. Patients requiring oxygen therapy or mechanical ventilation, ARDS (oxygenation index < 200mmHg) was diagnosed according to the Berlin standard in 2012.

Inclusion criteria: 1. Diagnosed Novel coronavirus pneumonia patients or clinically diagnosed NCP patients in Hubei Province;
2. Aged 18 to 65 years;
3. The time interval between symptom onset and randomization was within 7 days (the onset of symptom was mainly determined by the time of fever);
4. Sign the written informed consent before the study.

Inclusion criteria: Patients with new type of coronavirus pneumonia admitted to the designated hospital of Jinan City have the same diagnostic criteria as "Pneumonitis diagnosis and treatment plan for new type of coronavirus infection (latest version)
during 2020.02-2020.05

Inclusion criteria: (1) aged 18-80 years old;
(2) severe (including critical risk factors) and critical patients with confirmed novel coronavirus pneumonia;
(3) blood CRP and ESR were more than twice higher than normal or TNFa and Il-6 were higher than normal;
(4) sign the informed consent.

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Inclusion criteria: Patients who were diagnosed as COVID-19 by nucleic acid test and were in accordance with the clinical classification of severe or critically illness. (refer to the clinical classification criteria in the pneumonia diagnosis and treatment program of novel coronavirus infection, General Office of the National Health Commission (trial version 4)).

Inclusion Criteria:

Clinically diagnosed with Corona Virus Disease 2019
Increased interleukin-6
Sign the informed consent

Inclusion criteria: Aged 2-65 years, with a BMI of 18-28, who underwent otolaryngological surgery. The patients were ASA grade I-III.

Inclusion Criteria:

Age: 18-80 years old, male and female;
Covid-19 confirmed cases;
Comply with any of the following:

Inclusion criteria: 1. Those who agree to take part in the test and sign the informed consent form voluntarily;
2. Adults aged >=18years old, male or female;
3. The inpatients with mild or common type of COVID-19 confirmed according to the diagnostic criteria "COVID-19 diagnosis and treatment plan - Sixth trial edition" issued by the National Health Commission.
4. Respiratory Specimens (including but not limited to sputum, nasopharyngeal swab and secretion of lower

Inclusion criteria: 1. Those who agree to take part in the test and sign the informed consent form voluntarily;
2. Adult aged >=18years old, male or female;
3. The inpatients with mild or common type of COVID-19 confirmed according to the diagnostic criteria "COVID-19 diagnosis and treatment plan - Sixth trial edition" issued by the National Health Commission.
4. Respiratory Specimens (including but not limited to sputum, nasopharyngeal swab and secretion of lower

Inclusion criteria: 1. The patient signed an informed consent form to participate in the study or convalescent plasma therapy;
2. patient aged >=18 years old;
3. COVID-19 patients diagnosed by PCR;
4. Nucleic acid positive within 72 hours before blood transfusion;
5. Pneumonia confirmed by imaging;
6. Hospitalization for the following reason:
Fever: axillary temperature>=36.7 degree C, or oral temperature>=38.0 degree C, or anal or ear

Inclusion criteria: 1. Aged 18 to 75 years old;
2. The diagnosis of new-type coronavirus pneumonia was confirmed by RT-PCR or gene sequencing according to the "Diagnosis and Treatment Scheme for Pneumonia of New Coronavirus Infection (Trial Version 6 and Updated)", and the clinical type was general (including severe high-risk factors) and severe;
 Note: Patients with common new type of coronavirus pneumonia (including severe high-risk factors); patients with

Inclusion criteria: 1. Novel coronavirus pneumonia is diagnosed met the fifth edition of the national health and Health Committee;
2. Respiratory rate in calm state >=30 times / min; OR Finger pulse oxygen saturation under rest and non oxygen absorption state <=93%; OR PO2/FiO2)<=300mmHg; OR PO2<60mmHg.

Inclusion criteria: 1. Male and female, older than 18 years;
2. Patients with confirmed COVID19;
3. Need to continue the antivirus therapy;
4. Agree to sign the informed consent;
5. Able to ingest, absorb and tolerate oral medication;
6. Agree to collect clinical samples;
7. Consent to contraceptive measures for sex within 7 days of the last oral medication.

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Inclusion criteria: 1. Conforming to the Chinese and Western Medicine Diagnostic Standards of patients with novel coronavirus pneumonia (COVID-19) in recovery period;
2. Aged between 18 and 65 years old;
3. Signing the generalized informed consent form (The intervention objects is incorporated in the clinical study as desired).

Inclusion criteria: 1. Confirmed cases: positive nucleic acid test;
2. Aged 25-65 years;
3. Asexual difference;
4. Clinical stage (light, common type);
5. Immunology standard: peripheral blood lymphocyte was higher than the lower limit of normal;

Inclusion criteria: (1) Patients with Novel coronavirus pneumonia diagnosed according to <diagnosis and treatment scheme for pneumonia of new coronavirus infection (trial version 5)>;
(2) Inpatients aged >18 years old;
(3) Patients Know the informed consent, voluntary participate in this clinical trial and sign informed consent.

Inclusion criteria: 1. Aged between 18 and 75 years;
2. In accordance with NCP criteria for severe and critical illness. Combined with the following epidemiological history, clinical manifestations, and pathogenic examinations to make a comprehensive judgment;
3. Will not participate in other drug clinical trials since the first day of Ulinastatin Injection admission;
4. Will sign written informed consent. Adolescents or patients who cannot sign informed consent must

Inclusion criteria: (1) aged between 18 and 75 years old;
(2) Common type patients with confirmed COVID-19 according to the diagnostic criteria of "novel coronavirus Infection Diagnosis and Treatment Plan (Trial 6th Edition)" issued by the National Health and Safety Commission;
(3) Patients and their families voluntarily participated in the study and signed an informed consent form.

Inclusion Criteria:

Age ≥18 years at time of signing Informed Consent Form;
Those who meet the diagnosis of severe new coronavirus pneumonia with laboratory confirmed infection with CoVID-19;

Inclusion criteria: 1. Patients aged from 18 to 80 years;
2. conformed to epidemic history;
3. fever and respiratory symptoms;
4. normal or decreased leukocyte count in peripheral blood; or decreased lymphocyte count;
5. conformed to imaging features of COVID-19 pneumonia;
6. positive for COVID-19 nucleic acid test or second-generation sequencing of pathogens;
7. serum KL-6 1500U/ml;
8. and conforms to any of the following:
(1) respiratory

Inclusion criteria: 1. Patients aged 18 or older, and meet the diagnostic criteria of Diagnosis and Treatment Scheme of Novel Coronavirus Infected Pneumonia published by the National Health Commission. Criteria for diagnosis (meet all the following criteria):
(1) With epidemiological history;
(2) Clinical manifestations (meet any 2 of the following): fever, normal or decreased white blood cell count or lymphopenia in the early stage of onset, and Chest radiology in the early

Inclusion criteria: 1. Aged >=18 years old;
2. The person, who is diagnosed as COVID-19 according to the 'Diagnosis and treatment of novel coronavirus pneumonia (trial edition 5)', is treated with de-isolation and meets hospital discharge criteria:
Temperature returned to normal over 3 days, respiratory symptoms improved obviously, two consecutive respiratory tract pathogenic nucleic acid detection negative (sampling time interval at least 1 day), can be removed from isolation

Inclusion criteria: (1) Aged 18 to 65 years old;
(2) Patients with mild or normal type who have been diagnosed with new type of coronavirus infection [in accordance with the diagnostic criteria of the Notice on Printing and Distributing New Coronavirus Pneumonia Diagnosis and Treatment Plan (Trial Implementation) issued by the National Health and Medical Commission];
(3) According to the National Health and Health Commission's antiviral treatment for more than 10 days, the COVID-

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Inclusion criteria: 1. Novel coronavirus history before entering the group:
(1) there were travel history or residential history in Wuhan area or other areas with local cases within 14 days before onset;
(2) During the 14 days before onset, there were patients with fever or respiratory symptoms from Wuhan or other areas with persistent local cases;
2. Clinical symptoms and signs;
3. Laboratory examination and chest CT

Inclusion criteria: 1. Aged 18 to 70 years old, inpatients, male or female;
2. Patients with severe COVID-19: confirmed cases shall be in compliance with guideline of "Diagnosis and Treatment Plan for COVID-19 (Version 7)" or updated versions.Confirmed cases can be defined if suspected cases have characteristic of following pathogeny or serology:
1) Detect nucleic acid of novel coronavirus positive by real-time fluorescence RT-PCR;
2) Have highly homologous to known

Inclusion criteria: 1. Patients diagnosed with COVID-19 [in accordance with the diagnostic criteria of the Notice on Printing and Distributing COVID-19 Diagnosis and Treatment Plan (Trial Implementation Version 6th) issued by the National Health and Medical Commission], and meets the discharge criterion;
2. Aged 18 to 75 years old;
3. Be consious;
4. Volunteer to participate in the study and sign the informed consent form, and be able to understand, receive

Inclusion criteria: (1) Aged >=18 years;
(2) 2019-nCoV pneumonia is diagnosed according to the standard of diagnosis and treatment;
(3) Patient's condition has reached the standard of desolation / discharge specified according to the diagnosis and treatment plan;
(4) According to the diagnosis standard, the syndrome differentiation of TCM is FeiPiQiXuZheng;
(5) Volunteer to participate in the study and sign informed consent

Inclusion criteria: 1. Patients are eligible for inclusion if they meet all of the following criteria within 5-7 days after the first admission to the ICU.
2. The patients met the diagnostic criteria of 2019-nCoV pneumonia (new coronavirus infection) pneumonia treatment plan (Fifth version), the National Health Office [2020] No.103.;
3. The primary disease is 2019 nCoV infection;
4. Respiratory distress, RR >=30 times/unit;
5. Under resting state, blood oxygen saturation

Inclusion Criteria:

Male and female, 18-75 years old
Patients previously diagnosed with novel coronavirus pneumonia: the course of illness is no more than 14 days; if the course of the disease was more than 14 days, patient meets one of the following

Inclusion criteria: (1) Meet the diagnostic criteria for patients with COVID-19 of the Proposal of Diagnosis and Treatment of COVID-19;
(2) Aged 18-85 years;
(3) To agree to participate in the study and sign the informed consent

Inclusion criteria: (1) Patients with mild, common, and severe neocoronary pneumonia;
(2) Those who meet the diagnostic criteria for insomnia with traditional Chinese medicine;
(3) Those who meet the diagnostic criteria for insomnia with western medicine;
(4) Those aged 18-75 years;
(5) Agree to receive treatment, and the patient or legal guardian signs the information through mobile phone software and voice authorization Consent;
(6) During this period, the

Inclusion criteria: Positive result in nucleic acid test, medical diagnosis is Corona Virus Disease 2019

Inclusion criteria: (1) Patients aged 18-80 years;
(2) Clear diagnosis of patients with severe COVID-19;
(3) Voluntarily join the trial and sign the informed consent.

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Inclusion criteria: (1) Healthy subjects with negative COVID-19 nucleic acid test and antibody test;
(2) Aged 18-70 years;
(3) Sign the informed consent and volunteer to participate in this study.

Inclusion Criteria:

Willing and able to provide written informed consent prior to performing study procedures
Age ≥18 years, and ≤75 years;

Inclusion criteria: 1. male and female aged 18-70 years;
2. positive result of test for 2019 novel coronavirus nucleic acid;
3. Clinical diagnosed with pneumonia;
4. onset within 7 days (severe cases can be delayed to 14 days);
5. the informed consent shall be signed by me or the authorized agent;
6. subjects who can take oral medication or who receive oral medication;
7. agree to collect clinical samples;
8. female subjects of child-bearing age are not

Inclusion Criteria:

All subjects participating in this clinical study must meet all of the following criteria:

According to the Diagnosis and Treatment Protocol for COVID-19 (Trial Version 5), under the

Inclusion Criteria:

Sign written informed consent;
Age ≥18 years;
Conforms to the NCP Critical and Critical Diagnostic Standards, namely "Pneumonitis Diagnosis and

Inclusion Criteria:

patients with severe COVID-19 pneumonia
willing to give informed consent

Inclusion criteria: Novel coronavirus pneumonia diagnosis and treatment plan (trial version seventh) was revised to meet the diagnostic criteria of the 18-65 year old patients

Inclusion criteria: 1. Mild or moderate cases who meet the "Diagnosis and Treatment Protocol for Novel Coronavirus Pneumonia" issued by the National Health And Health Council are trialled in the fifth and sixth editions of clinical diagnosis and diagnostic criteria;
2. Aged >=18 years;
3. Patient informed consent.

Inclusion Criteria:

Aged 18-65 years;
Voluntarily participate in this clinical trial and sign off "informed consent form";
Diagnosed with severe pneumonia of COVID: respiratory distress, RR >30 times / min; resting

Inclusion criteria: 1) Aged 18 to 70 years old;
2) Infection with new coronavirus pneumonia was confirmed by throat swab nucleic acid test. Patients with severe and critical illness were clinically judged. The symptoms of acute infection have been alleviated after treatment (at least 14 days after onset, no fever for more than 3 days), and the disease is recovering Within three months;
3) CT examination of patients with multiple fibrotic shadows in both lungs;
4) Blood routine, liver, and

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Inclusion criteria: 1. Aged 18 to 75 years old;
2. mild or normal patients;
3. not using chloroquine phosphate, lopinavir other than other antiviral drugs;
4. under the gastroscope, there was mucosal damage and pathological biopsy of 2019-ncov nucleic acid positive;
5. confirmed cases meeting all the following criteria (the 5th edition of the medical guidelines):
(1) epidemiological history;
(2) Clinical manifestations (in accordance with any 2 of the following):

Inclusion Criteria:

COVID-19 has been diagnosed, and the nucleic acid test of respiratory specimens such as sputum or nasopharyngeal swabs has been negative for two consecutive times after treatment (sampling time interval of at least 24 hours);

Inclusion Criteria:

Confirmed COVID-19, and the nucleic acid test of respiratory specimens such as sputum or nasopharyngeal swabs is negative twice after the treatment (sampling interval is at least 24 hours); Negative nucleic acid test of respiratory specimens such as sputum or nasopharyngeal swabs

Inclusion criteria: (1) patients with definite diagnosis of novel coronavirus pneumonia and hospitalized at Leishenshan hospital;
(2) patients aged 18-80 years old;
(3) patients who signed informed consents, voluntarily and persistently participated in the study and follow-up.

Inclusion criteria: 1. Meet the history of COVID-19 confirmed in accordance with the diagnosis and treatment plan of COVID-19(trial seventh edition);
2. Meet the discharge standards of COVID-19(trial seventh edition);
3. Meet the diagnosis standard of qi deficiency and phlegm stasis syndrome;
4. Patients aged 18-70 years old, 28 days after discharge, still have abnormal manifestations on lung CT;
5. Patients willing to sign informed consent

Inclusion Criteria:

Aged between 18 and 60 years.
Able to understand the content of informed consent and willing to sign the informed consent
Able and willing to complete all the secluded study process during the whole 6 months study follow-

Inclusion criteria: 1. Aged 18 years old;
2. diagnosed with viral pneumonia such as new coronavirus;
3. voluntary informed consent

1. Healthy subjects aged 6 years and above;
2. Request medical history and physical examination, and the researchers judged health status of subjects;
3. Since December 2019, subjects have not been to Hubei, overseas and villages/communities with COVID-19 epidemic, and do not contact with COVID-19 cases or suspected cases. Subject shall be

Inclusion criteria: 1. Discharged patients with COVID-19 pneumonia;
2. With dyspnea symptom (mMRC 2~3);
3. Aged 18~75 years old;
4. Have and can use smart phone (study subjects or family members);
5. Obtain the consent of the subjects or legal representative and sign the informed consent.

Inclusion criteria: 1. Patients who have been diagnosed as COVID-19 according to The Diagnosis and Treatment Plan for 2019-nCoV(The Fifth Trial Edition);
2. aged between 18 to 80 years old, male or female;
3. the subjects participating the clinical trial are voluntary and sign the informed consent, thoroughly informed the purpose, method, possible reaction.

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Inclusion Criteria:

Healthy adults aged 18-59 years;
Proven legal identity;
Participants should be capable of understanding the informed consent form, and such form should

Inclusion criteria: (1) Both male and female, 18 years old <= aged <= 75 years old;
(2) The territory is limited to wuhan, hubei province;
(3) Voluntarily participate in the study and sign the informed consent.

(1) novel coronavirus pneumonia has not been stopped since the outbreak of COVID-19;
(2) the co worked personnel were diagnosed as COVID-19 or asymptomatic infection or SARS-CoV-2 IgM positive within 14 days;
(3) the detection of nucleic acid, IgM and IgG of SARS-CoV-2 was negative within 1 weeks;
(4) between 18-70 years old;

Inclusion criteria: 1. Aged >=18 years old;
2. Patients suspected of having COVID-2019 pneumonia who also have one of the following etiological or serological evidence:
(1) Real-time fluorescent RT-PCR detection of COVID-2019 nucleic acid positive;
(2) viral gene sequencing, and Known novel coronaviruses are highly homologous;
(3) Serum novel coronavirus-specific IgM antibodies and IgG antibodies are positive; serum novel coronavirus-specific IgG antibodies

Inclusion criteria: 1) Aged 18 to 75 years old, male or female;
2) The confirmed participants were diagnosed as mild, severe or critical cases according to the notice on printing and distributing the diagnosis and treatment plan of pneumonia with new coronavirus infection (trial version 7);
3) Those who agree not to participate in other clinical studies within 30 days after administration of the study;
4) Patients voluntarily signed informed consent

Inclusion Criteria:

Clinical diagnosis of the convalescence of COVID-19 (defined by the Chinese Guideline for COVID-19 in 2020 Trial Version 7).
18-70 years old;

Inclusion criteria: 1. Patients who meet the COVID-19 diagnostic criteria and discharge standards with SARS-CoV-2 virus nucleic acid test is negative;
2. The pulmonary imaging in the last 1 week showed inflammation or fibrosis;
3. Aged 18 to 70 years old;
4. Signed informed consent.

Inclusion criteria: 1. Meet the convalescence of COVID-19 diagnostic criteria;
2. Short breath, hyperhidrosis, dry cough, chest distress, palpitation and other clinical discomfort symptoms of lung and heart function decline, in which 3 symptoms appear at the same time, or the visual analogue score of single symptom (VAS) >=4 points;
3. Patients who were discharged for more than 2 weeks were cured;
4. Aged 18-70 years old;
5. Sign informed consent

Inclusion criteria: 1. COVID-19 convalescent diagnostic criteria;
2. Cured and discharged patients more than 2 weeks;
3. Aged from 18 to 70 years;
4. Those with insomnia, anxiety, depression and other sleep mood disorders as the main clinical manifestations, and those with two symptoms at the same time, or visual analogue scale (VAS) of single symptom >=4;
5. Sign the informed consent

Inclusion criteria: 1. The subject or legal agent has signed the informed consent form; agreed to not participate in other clinical researchers within 30 days from the first administration of the study drug to the last administration.
2. Aged 18 to 75 years old;
3. Meet the diagnostic criteria for pneumonia caused by new coronavirus (2019-nCoV) (seventh edition)
4. Treated patients or relapsed patients meet any of the following:
(1) Fever again, or the clinical symptoms

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Inclusion Criteria:

The patient volunteered to participate in the study, approved the aspirin treatment, and was willing to randomly accept one of the aspirin treatment regimens, and provided written informed consent,

Subject is required to meet one of the following criteria for confirmation of a novel coronavirus

Inclusion criteria: (1) Convalescent patients with novel coronavious pneumonia,and the patient is aged 18-75 years old;
(2) The TCM syndrome type conforms to deficiency of 'qi' and 'yin' and not exhausted of 'Yu Xie';
(3) Voluntary test, or the immediate family or the client's approved client agrees to the test, and voluntarily signs the informed consent;
(4) Provide detailed contact information, no short-term willingness to move and be willing to cooperate with followers

Inclusion criteria: 1. COVID-19 confirmed with SARS-COV-2 RNA detection;
2. Hypertension diagnosis;
3. Aged >=18 years.

Inclusion criteria: (1) There is no restrictions on gender and age;
(2) Patients diagnosed with mild pneumonia (slight clinical symptoms, no pneumonia manifestations on imaging), general pneumonia patients (with fever, respiratory tract symptoms, etc., imaging showed pneumonia but no multiple organ damage);
(3) Hospitalized patients;
(4) Volunteer to join the trial and sign the "informed consent;
(5) Promise not to perform other exercise activities

Inclusion criteria: 1. COVID-19 convalescent diagnostic criteria;
2. Cured and discharged patients more than 2 weeks;
3. Aged from 18 to 70 years;
4. Patients with sleep mood disorders such as restlessness, anxiety and sleep disorders were the main clinical manifestations, and those with two symptoms at the same time, or visual analogue scale (VAS) of single symptom >=4;
5. Sign the informed consent

Inclusion criteria: 1. Patient diagnosed with COVID-2019(positive for swab or sputum nucleic acid test);
2. The patients meet the definition of COVID - 19 (seventh edition) in the national health commission of the general and severe type;
3. The patients were in recovery;
4. The age of patients ranged from 18 to 70 years old.

Inclusion criteria: 1. Healthy subjects aged 3 years and above;
2. By asking for a medical history and a physical examination, the researchers determined that the subjects were in good health;
3. From December 2019 to now, the subject has not been to hubei province, outside the country or in a village/community where there has been an outbreak. The subject has not been exposed to a person infected with or suspected of COVID-19;
4. Female subjects with

Inclusion criteria: 1. Healthy subjects with negative covid-19 nucleic acid test and antibody test;
2. Aged 18-70 years old;
3. Signed informed consent and participated in the study voluntarily.

Inclusion criteria: 1. Aged >= 18 years, male or female.
2. Patients who were diagnosed as COVID-19 according to the The Diagnosis and Treatment Scheme of COVID-19(The 7th Trial Edition).
3. The condition meets the discharge standards stipulated in the The Diagnosis and Treatment Scheme of COVID-19(The 7th Trial Edition) after treatment.
4. The syndrome differentiations are the deficiency of lung and spleen qi at the time of screening. According to the

Inclusion criteria: (1) Patients who meet the COVID-19 diagnostic criteria and discharge standards, but the later SARS-CoV-2 virus nucleic acid test is positive;
(2) Aged 18 to 70 years old;
(3) Signed informed consent.

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Inclusion criteria: 1. COVID-19 patients accompanied with gastrointestinal symptoms;
2. COVID-19 patients accompanied with antibiotic-associated diarrhea

Inclusion criteria: (1) age =18 years old, gender not limited;
(2) Laboratory (RT-PCR) confirmed COVID-19;
(3) the time from the first positive nucleic acid test to randomization does not exceed more than 4 days;
(4) informed consent has been signed.

Inclusion Criteria:

Healthy adults aged ≥60 years;
Be able to understand and sign the informed consent voluntarily;
Provide legal identification;

Inclusion Criteria:

Patients confirmed with COVID-19 and classified as severe

Inclusion criteria: 1. Aged 2 to 65 years old, and signed informed consent;
2. According to the diagnostic criteria of the "New Coronavirus Pneumonia Diagnosis and Treatment Program (Trial Version 7)" issued by the National Health and Health Commission, those who have been diagnosed with CoVID-19 of light and common type and have been treated with hydroxychloroquine and Abidol but nCoV Nucleic acid did not turn negative. The specific diagnostic criteria are as follows:
(1)

Inclusion criteria: 1. Meet the diagnostic criteria of the n Novel Coronavirus Pneumonia (COVID-19) in the recovery period;
2. Patients with fatigue, anorexia, abdominal distension, loose stools and other digestive dysfunction as the main clinical manifestations, 3 of them at the same time, or a single symptom visual analog score (VAS) greater than 5 points;
3. Heal patients who have been discharged for more than 2 weeks;
4. Aged 18-70 years old;
5. Sign the informed

Inclusion criteria: 1. According to the standard of the Chinese version New Coronavirus Pneumonia Diagnosis and Treatment Program (5th edition), the participant is clearly diagnosed as Coronavirus Disease 2019 (COVID-19) and classified as mild or common type;
2. During treatment in hospital, the condition tends to be stable;
3. Write the informed consent form and be able to insist on long-term follow-up

Inclusion criteria: 1. Patients with confirmed new coronavirus-infected pneumonia;
2. Aged >=18 years old;
3. Voluntarily sign written informed consent.

Inclusion Criteria:

Phase Ia:

Healthy adults aged 18 to 59 years (including boundary values), both men and women

Inclusion criteria: 1. Patients have been confirmed diagnosis from a positive test for COVID-19 RT-PCR,>18 years old;
2. Patients who were classified as mild or common clinical types according to the Diagnosis and Treatment Protocol for Novel Coronavirus Infection-Induced Pneumonia version 5 (trial);
3. Patients who were able to skillfully use the mobile phone app.

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Inclusion criteria: 1. The novel coronavirus pneumonia, once diagnosed as a severe or critical illness, has been cured and discharged without any pathogen or rehabilitation;
2. It conforms to the above diagnostic standards of Western medicine and TCM syndrome differentiation;
3. The lung function indexes meet the following standards:
(1) the percentage of forced vital capacity in predicted value (FVC%) is between 50% - 90%;
(2) the percentage of carbon monoxide

Inclusion criteria: 1. Adults (defined as those aged >=18 years) ;
2. The CT findings of patients with novel coronavirus infection or pneumonia confirmed by PCR;
3. In-patients, accompanied by fever (Temperature at Armpit > 37 degree C) or cough, dyspnea;
4. The interval between symptom onset and randomization was less than 8 days;
5. Agree to participate in the study and accept the randomization;
6. Who has not participated in a drug clinical trial within 3

Inclusion criteria: (1) There is no restrictions on gender and age;
(2) Patients diagnosed with mild pneumonia (slight clinical symptoms, no pneumonia manifestations on imaging), general pneumonia patients (with fever, respiratory tract symptoms, etc., imaging showed pneumonia but no multiple organ damage);
(3) Hospitalized patients;
(4) Volunteer to join the trial and sign the "informed consent;
(5) Promise not to perform other exercise activities

Inclusion criteria: 1. Conform to the diagnostic criteria of Western Medicine's New Coronary Pneumonia;
2. Aged 18-65 years old male or female.

Inclusion Criteria:

Male and female subjects aged 18 to 45 years, inclusive;
The body weight no less than 50 kg for male subjects and no less than 45 kg for female subjects.
Body mass index (BMI) = weight (kg)/square of height (m2), ranging from 18-28 kg/m2 (including the

Inclusion criteria: 1. Aged between 2 and 18 years male or female;
2. Laboratory (RT-PCR) and clinical symptoms confirmed case of COVID-19, according to the official guideline "Pneumonia Diagnosis and Treatment Scheme for Novel Coronavirus Infection (Trial Version 6) ". Or the patients with negative 2019-nCoV RNA results but clinical symptoms suspected;
3. The legal guardian of the enrolled children shall sign the informed consent voluntarily (subjects >=8 years old shall sign

Inclusion Criteria:

Comply with the observation age of this clinical trial: 18-59 years old (both included) adults.
The subjects themselves voluntarily agreed to participate in the study, and signed an informed consent form, and can provide legal identification; understand and abide by the requirements of the

Inclusion criteria:
 1. Patients with COVID-19 who had mild symptoms in line with the diagnostic criteria of the Chinese Management Guidelines for COVID-19 (version 6.0)
2. Good communication and understanding of Chinese
 3. Aged 20-69 years

Inclusion criteria: 1. Healthy adults aged 18-59 years, or 60 years and above who can provide legal identity certificate of their own, both males and females are acceptable;
2. Subjects who understand the contents of the informed consent form and the background of the investigational vaccine, agree to sign the informed consent form voluntarily, and are able to use thermometer and scale, and may fill in the diary card and contact card as required;
3. Subjects who are able to

Inclusion Criteria:

Persons with full civil capacity aged 18-59 years (both included);
The subjects themselves voluntarily agreed to participate in the study, and signed an informed consent form, and can provide valid identification; understand and comply with the requirements of

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Phase Ib:

Inclusion Criteria:

Subject must be 18 to 49 years of age inclusive;
Body weight ≤100 kg and body mass index (BMI) within the range of 19.0-24.0kg/m2 (inclusive);
Male or female;
Inclusion Criteria:

Subject must be 18 to 49 years of age inclusive;
Body weight ≤100 kg and body mass index (BMI) within the range of 19.0-24.0kg/m2 (inclusive);
Male or female;
Inclusion Criteria:

Able to provide written informed consent
Males or females. Aged ≥ 18 years old
Body mass index (BMI) between 18.0 and 26.0 kg/m2
Inclusion criteria: 1. Patients who met the discharge standards with the criteria for Diagnosis and Treatment Guideline of Novel Coronavirus Pneumonia (Trial 7th edition) and have a negative nucleic acid test for the new coronavirus after discharge
2. lung CT showed inflammation in the lungs or manifestations of pulmonary fibrosis in the past 1 week;
3. Aged 18-70 years old;
4. Sign informed consent
Inclusion criteria: 1) The subject fully understands the purpose, nature, method and possible adverse reactions of the study, volunteers to be a subject, and signs the informed consent prior to the commencement of any study procedure, and warrants that he/she will participate in any study procedure;

2) The subject can have good communication with the researcher, and understand and comply standard constrain ;

Persons with full capacity for civil conduct aged ≥60 years;
Subjects voluntarily agree to participate in the research and sign an informed consent form, and can provide valid identification; understand and comply with the requirements of the trial protocol; have
Inclusion Criteria:

For adult group (age ≥18 and ≤55)

Male or female subjects of ≥18 years old and ≤55 years old with body mass index (BMI) ≥18 and
Inclusion Criteria:

Subjects are fully aware of the purpose, nature, methods and possible adverse reactions of the study, voluntarily serve as subjects, and sign the informed consent form before the start of any study procedures, and ensure that he/she will participate in any procedures in the study in person;
Inclusion Criteria:

Male or female subjects of ≥ 18 years old with body mass index (BMI) ≥ 18.5 and ≤ 30 at the Screening Visit.
The subject can provide with informed consent and signs and dates a written informed consent form

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Inclusion criteria: 1. Subjects over 18 years old;
2. Patients with novel coronavirus pneumonia history;
3. Those who can read Chinese characters;
4. Those who can independently cooperate with doctors to complete various scales and corresponding tests;
5. Subjects with mobile communication equipment such as mobile phones or tablets;
6. Mobile equipment can be connected to the Internet at any time;
7. The patient himself had informed consent in the

Inclusion criteria: 1) Aged >=18 years old at the time of enrollment;
2) Be able to comply with the requirements of the clinical research protocol, complete all trial procedures, and sign the informed consent form;
3) The nucleic acid test of SARS-CoV-2 is negative;
4) The serum total antibody test of SARS-CoV-2 is negative;
5) The lung CT imaging is normal (no imaging features of COVID-19);
6) The HIV screening result is negative;
7) For female subjects,

Inclusion criteria: 1. Male or female volunteers, age: 18-59 years old (including threshold).
2. Body weight: male >= 50 kg, female >= 45 kg; body mass index (BMI) in the range of 18-30 kg / m2 (including critical value).
3. From signing the informed consent form to 12 months after the last vaccine injection, the fertile female subjects or partners with fertile male subjects agreed and could take effective contraceptive measures, such as avoiding sexual life or using oral contraceptives

Inclusion Criteria:

Healthy children and adolescents aged 3-17 years;
The subject and/or guardian can understand and voluntarily sign the informed consent form (double sign required for 8-17 years old);

Inclusion criteria: 1. Patients who meet the high risk assessment criteria for avascular necrosis of the femoral head after recovery of patients with new coronavirus pneumonia;
2. Patients aged 18-70 years old;
3. Patients who voluntarily participate in clinical research and sign informed consent.

Inclusion criteria: 1. Meet the high risk assessment criteria of avascular necrosis of femoral head in patients with new coronavirus pneumonia after recovery;
2. Patients aged 18-70 years old;
3. Patients who voluntarily participate in clinical research and sign informed consent.

Inclusion Criteria:

Aged 18 to 55 years old in MID group; aged 6-17 years old in MIN group; aged 56 years and above in OLD group; aged in EBO group is not limited.
Able to understand the content of informed consent and willing to sign the informed consent

Inclusion Criteria:

Age of 18-65 years (inclusive) ,women or man
SARS-CoV-2 detected in the diagnostic specimen (nasopharyngeal swab)
High homology of viral gene sequencing with the known SARS-CoV-2 "

Inclusion criteria: 1. Patients who meet the recovery period of new coronavirus pneumonia;
2. Syndrome differentiation is of Qi and Yin deficiency type;
3. Informed consent to this study, voluntarily participate in clinical research;
4. Can provide detailed contact information, have no short-term relocation wishes, and are willing to cooperate with follow-up.

1. Healthy permanent residents aged 18 years and above;
2. Subjects agree to sign the informed consent forms voluntarily;
3. Subjects are able to comply with the requirements of the clinical trial protocol;
4. Armpit temperature <=37.0 degree C;
5. Female subjects of childbearing age were not pregnant at the time of enrollment, were not

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Inclusion Criteria:

1) Male or female, aged between 18 and 75 (including boundary values) at screening.

2) Severe or critically ill patients who have been diagnosed with a novel coronavirus during

Inclusion criteria: 1. Healthy adults aged 18-59 who can provide the legal identification of the subjects, both male and female;
2. The subjects knew the content of the informed consent and the vaccination situation, voluntarily signed the informed consent, and had the ability to use the thermometer, scale and fill in the diary card and contact card as required;
3. Those who can communicate well with researchers, and understand and comply with the requirements of this study;

Inclusion criteria: The inclusion criteria of study participants included:
1. According to the diagnosis and clinical management of New Coronavirus infection in 2019 (trial version 6 or trial version 7), COVID-19 was diagnosed;
2. Patients with pcl-c score >= 50;
3. Patients willing to participate in the study.

Inclusion criteria: 1. Healthy permanent residents aged 18 years and above;
2. Subjects agree to sign the informed consent forms voluntarily;
3. Subjects are able to comply with the requirements of the clinical trial protocol;
4. Armpit temperature <= 37.0 degrees C;
5. Female subjects of childbearing age were not pregnant at the time of enrollment, were not breastfeeding, and had no birth plan within the first 3 months after enrollment; effective

Inclusion Criteria:

Healthy adults aged 18-59 or elderly aged ≥60 years;
Proven legal identity.

Inclusion criteria: 1. Subjects over 18 years old at the time of enrollment;
2. The subjects who can comply with the requirements of the clinical study protocol, complete all the trial procedures, and sign the informed consent form;
3. Since December 2019, the subjects who have not been to the epidemic area, have not contacted the new coronal infection or suspected cases, and are in the non isolation period, and there are no new coronal infection or suspected cases in the same village /

Inclusion Criteria:

Patients diagnosed with mild or moderate COVID-19 (as per the Diagnosis and Treatment Protocol for Novel Coronavirus Pneumonia (Trial, 8th edition), China, or Clinical Management of COVID-19, WHO).

Inclusion Criteria:

Male or female participants between the ages of 18 and 85 years, inclusive, at randomization.
Participants who are willing and able to comply with all scheduled visits, vaccination plan, laboratory tests, lifestyle considerations, and other study procedures.

Inclusion Criteria:

Healthy subjects of ≥ 18 years old.
The subject can understand and voluntarily sign the informed consent.
The subject can The subject canprovide legal identification.

Inclusion Criteria:

Aged 18 years and above.
Able to understand the content of informed consent and willing to sign the informed consent.
Able and willing to complete all the secluded study process during the whole study follow-up period.

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Inclusion criteria:

1.Age >= 18 years old and < 60 years old, regardless of gender (for adult subjects only);

2.Age >= 60 years old and <= 85 years old, regardless of gender (for elderly subjects);

3.Body mass index (BMI) is between 18-35 kg/m2 (including upper and lower limits);

4.Axillary temperature < 37.3 degrees C on the day of vaccination

Inclusion criteria: 1. People who are 18 years old and above;
2. The subjects themselves voluntarily participate in the study, sign an informed consent form, and can provide valid identification, understand and comply with the requirements of the trial protocol;
3. Female subjects of childbearing age agree to take effective contraceptive measures from the beginning of the study to 2 months after the full vaccination

Inclusion Criteria:

Patients with mild and severe COVID-19 confirmed according to the Diagnosis and Treatment Protocol for Novel Coronavirus Pneumonia (Trial Version 7) that was issued by the National Health Commission & State Administration of Traditional Chinese Medicine;

Inclusion criteria: 1. Aged from 18 to 70 years old;
2. The diagnostic criteria of subjects clearly defined as "SARS-CoV-2 infected pneumonia" shall be based on the criteria of severe patients stipulated in the "Novel Coronavirus Infected Pneumonia Protocol (Trial version vii)" issued by the National Health Commission;
3. Patients can receive aerosol inhalation;
4. Consent to sign a written informed statement prior to the beginning of the study

Inclusion Criteria:

Subject must be 18 to 49 years of age inclusive

Body weight ≤100 kg and body mass index (BMI) within the range of 19.0-26.0kg/m2 (inclusive).

Male or female

Inclusion criteria: 1. Subjects aged 18-59 years;
2. The subjects who signed the informed consent form and participated voluntarily;
3. Subjects with good compliance;
4. Subjects with the interval of more than 14 days from the last vaccination;
5. Before entering this study, subjects with history and clinical examination and body temperature <= 37.0 degrees C.

Inclusion criteria: 1. Subjects aged 18-59 years old;
2. The subjects who signed the informed consent form and participated voluntarily;
3. Subjects with good compliance;
4. Subjects with the interval of more than 14 days from the last vaccination;
5. Before entering this study, subjects with history and clinical examination and body temperature <= 37.0 degrees C.

Inclusion criteria: 1. Subjects aged 18-59 years;
2. The subjects who signed the informed consent form and participated voluntarily;
3. Subjects with good compliance;
4. Subjects with the interval of more than 14 days from the last vaccination;
5. Before entering this study, subjects with history and clinical examination and body temperature <= 37.0 degrees C.

Inclusion criteria: 1. A healthy person aged 18-59 who can provide the legal identification of the subjects, both male and female;
2. The subjects who knew the content of informed consent and the vaccination situation voluntarily signed the informed consent, and had the ability to use the thermometer, scale and fill in the diary card and contact card as required;
3. Subjects who can communicate well with researchers and understand and comply with the requirements of this study;

1. Patients with confirmed COVID-19 by Pathogen testing;

2. Above 18 years old (inclusive), both male and female;

3. Severe novel coronavirus pneumonia meets one of the following conditions;

(1) Shortness of breath, RR>=30 times/min;

(2) In the resting state, the oxygen saturation is less than or equal to 93% when inhaling air;

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1. Patients with confirmed COVID-19 by Pathogen testing;
2. Above 18 years old (inclusive);
3. At the time of enrollment, the patient had any of the main symptoms (fever, cough, expectoration, chest tightness, polypnea, dyspnea);
4. Voluntarily sign written informed consent

1. Asymptomatically infected with novel coronavirus pneumonia discovered through pathogenic testing;
2. Over 18 years old (including 18 years old);
3. Voluntarily sign a written informed consent before the study begins.

Inclusion criteria: 1. Male and female subjects aged 18 to 45 years, inclusive;
2. The body weight no less than 50 kg for male subjects and no less than 45 kg for female subjects. Body mass index (BMI) = weight (kg)/square of height (m2), ranging from 18-28 kg/m2 (including the critical value);
3. These indicators are normal including physical examination, vital signs, laboratory tests and other accessory examinations (chest radiology, abdominal B-mode ultrasonography, ECG

Inclusion Criteria:

Aged 18-85 years old
Able to understand the content of informed consent and willing to sign the informed consent.
Able and willing to complete all the secluded study process during the whole study follow-up period

1) Patients who met the diagnostic criteria of covid-19 and discharged after treatment;
2) Aged 18-75 years;
3) Having one or more clinical manifestations of respiratory symptoms or fatigue;
4) Patient informed consent.

Inclusion criteria: 1. Confirmed as a close contact with the novel coronavirus pneumonia through flow investigation;
2. Over 12 years old (including 12 years old), no gender limit;
3. Voluntarily sign a written informed consent before the start of the study.

Inclusion criteria: 1. Meet the diagnostic criteria for the recovery period of new coronavirus pneumonia;
2. Aged from 18 to 70 years old;
3. Those who have depression, anxiety, poor sleep, fatigue and other sleep mood disorders as their main clinical manifestations, and those who have 2 symptoms at the same time;
4. Screening and baseline Hamilton Depression Scale (HAMD) 17 points >=17 points, <=24 points;
5. Patient Health Questionnaire PHQ-15>=5

Inclusion Criteria:

Subject ≥ 18 years, signing the informed consent.
SARS-CoV-2 infection by PCR ≤ 7 days
One or more of COVID-19 related symptoms or measured fever present within 48 hours prior to

1. Request medical history and physical examination, and the researchers judged health status of subjects;
2. Aged 18 to 60 years;
3. BMI 18.5-30Kg/m2;
4. Novel Coronavirus (COVID-19) antibodies (IgG, IgM) are negative;

Inclusion Criteria:

Healthy adults aged 18-59 years;
The subject can understand and voluntarily sign the informed consent form;
Proven legal identity

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Inclusion Criteria:

Healthy subjects of ≥ 18 years old.
The subject can understand and voluntarily sign the informed consent.
Axillary temperature <37.0℃
For the diagnosis of light and common COVID-19, refer to COVID-19 Diagnosis and Treatment Protocol (Trial Eighth Edition) (hereinafter referred to as "Diagnosis and Treatment Protocol");
Reference for asymptomatic infected persons: COVID-19 Control Plan (7th Edition).
1. Conform to the diagnostic criteria of mild, common COVID-19 and novel Coronavirus asymptomatic infection in the two protocols mentioned above;
Inclusion Criteria:

The subjects ≥ 18 years old who has completed one dose of recombinant Ad5 vectored COVID-19 vaccine.
The subjects can provide with informed consent and sign informed consent form (ICF)
Subjects who comply with the following criteria will be enrolled:
1. Willing to participate in the study with informed consent;
2. Health male or female adults (i.e. aged 18 years and above);
3. Without a history of traveling or residence in domestic areas of high and moderate pandemic risk, overseas or epidemic areas, nor a history of contact with confirmed, asymptomatic or suspected
Subjects who comply with the following criteria will be enrolled:
1. Willing to participate in the study with informed consent;
2. Health male or female adults (i.e. aged 18 years and above);
3. BMI within 18-28 kg/m2 (including both boundaries);
4. Without a history of traveling or residence in domestic areas of high and moderate pandemic risk

Aged 18-59 years.

Inclusion Criteria:

Healthy adults 18 years of age and above at the time of enrollment;
Able to provide consent to participate in and having signed an Informed Consent Form (ICF);
Able and willing to complete all the scheduled study procedures during the whole study follow-up
Inclusion Criteria:

Age range: healthy people aged 3 years and and older who can provide legal identification;
The subject and/or his legal guardian and/or his entrusted person can understand the study procedures and informed consent, voluntarily sign informed consent form, and be able to comply
Inclusion Criteria:

Aged 3 years and above (after enrolled, subjects will be allocated according to age).
By asking for medical history and physical examination, the health condition judged by the investigators is well
1. Chinese aged 18 or above who can provide legal proof of identity;
2. According to the medical history and physical examination, the researcher determined that the patient was healthy, both male and female;
3. Having the ability to understand the study procedures, have informed consent, voluntarily signed informed consent, can comply with the requirements of the clinical study protocol, having the ability

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Inclusion Criteria:

Healthy children and adolescents aged 3-17 years;
The subjects and/or guardians can understand and voluntarily sign the informed consent form (For subjects aged 8-17 years, both subjects and guardians need to sign the informed consent form);

1. Aged >=18 years old when enrolled;
2. Participants signing the informed consent;
3. Being able to participate in all planned follow-ups and accomplish all research procedures including completing a diary card and coming back for interview;
4. Having not been administered any pneumonia vaccine or COVID-19 vaccine, and having not

1. Aged >=18 years when enrolled;
2. Participants signing the informed consent;
3. Being able to participate in all planned follow-ups and accomplish all research procedures including completing a diary card and coming back for interview;
4. Having not been administered any pneumonia vaccine or COVID-19 vaccine, and having not

1) Conform to the diagnostic criteria of COVID-19 (Guidelines for the diagnosis and treatment of coronavirus disease 2020(trial version eighth)) Patients who met the discharge standard and had negative neocoronavirus nucleic acid detection after discharge
2) Aged 18-75 years;
3) Having clinical manifestations of respiratory symptoms or fatigue;

Inclusion Criteria:

the patients diagnosed as the common or severe cases of COVID-19
aged 18-85
the absolute lymphocyte counts below the normal value (≤1.1-3.2×109/L)

Inclusion Criteria:

6-59 years of age at the time of enrollment;
Able to provide consent or obtain consent from guardian to participate in the trial and sign an Informed Consent Form (ICF);

Inclusion Criteria:

Health subjects aged 18-59 years, who have been completed two-dose regimen of inactive SARS-CoV-2 vaccine in the past 3-6 months, or received one dose of inactive SARS-CoV-2 vaccine in the past 1-3 months

1) Be at least 18 years old at the time of entry;
2) Able to comply with the requirements of the clinical research plan to complete all trial procedures and sign the informed consent form;
3) For female subjects: infertility (surgical sterilization or at least one year after menopause) or willing to use appropriate contraceptive measures from 30 days before vaccination to 3 months after

1. Aged over 18 years;
2. Received two doses of the new crown inactivated vaccine at least 4 months ago;
3. According to the medical history and physical examination, the researchers judged to be healthy (excluding the following conditions: known or suspected to suffer from more serious diseases at the same time, including: respiratory diseases, acute infections or active chronic diseases, liver and

1. Meet the diagnostic criteria for COVID-19 (National Eighth Edition);
2. Aged over 18 years;
3. Normal body temperature within 48 hours;
4. In the case of inhaling air, SpO2>93%;
5. The onset time is more than 7 days;

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Inclusion Criteria:

Male or female ≥18 years of age.
Individuals are willing and able to comply with study requirements, including all scheduled visits, vaccinations, laboratory tests and other study procedures

Inclusion Criteria:

A signed informed consent form (ICF) from the patient or their legally authorized representative.

Inclusion Criteria:

Healthy males or females, ages ≥18 to ≤45 years. Male subjects with weight ≥50 kg, or female

Inclusion Criteria:

Healthy population aged 18 years and above;
The subjects can understand and voluntarily sign the informed consent form;
Proven legal identity

Inclusion Criteria:

Aged 3-17 years old (all inclusive);
The subject voluntarily agrees to participate in the study (or the legal guardian of the subject voluntarily agrees the child to participate in the study), and the guardian and the subject (8-17 years

Inclusion Criteria:

Healthy individuals aged 18-59 years as well as 60 years and above who can provide legal identification (males and females are both required).
Willing to participate in the study with informed consent prior to screening

Inclusion Criteria:

Healthy individuals aged 18-59 years as well as 60 years and above who can provide legal identification (males and females are both required).
Willing to participate in the study with informed consent prior to screening

Inclusion Criteria:

Healthy adults aged 18 years and above;
Participants should be capable of understanding the informed consent form, and such form should be signed prior to enrolment .

Inclusion Criteria:

Healthy residents aged 3 to 17 years at the time of consent
Subjects and/or their guardian agree to sign the informed consent forms voluntarily.
Be able to comply with study requirements/procedures

Inclusion Criteria:

Healthy residents aged 3 to 17 years at the time of consent
Subjects and/or their guardian agree to sign the informed consent forms voluntarily.
Be able to comply with study requirements/procedures

Inclusion Criteria:

Healthy adults aged 26-45;
The subjects can understand and voluntarily sign the informed consent form ;
Proven legal identity

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Inclusion Criteria:

Aged 6-17 years ;
The subject and/or guardian can understand and voluntarily sign the informed consent form (double signature is required for 8-17 years old) ;

1. People aged 18 and above;
2. The subject has independent judgment, can read, understand and complete the vaccination diary card, and voluntarily participate in the vaccination and sign the informed consent form;
3. The subjects can comply with the requirements of the clinical trial protocol;
4. Armpit body temperature <=37.2 degree C;

(1) Diagnostic criteria of discharged patients who met the COVID-19 Diagnosis and Treatment Protocol (Trial Version 6 and Version 7);
(2) Patients who meet the TCM diagnostic criteria of deficiency of lung and spleen and deficiency of Qi and Yin;
(3) Aged between 18 and 65 years;

(1) Diagnosis of light or common type of delta novel coronavirus pneumonia (COVID-19) .The onset was within 7 days.
(2) Informed consent, voluntary test; The process of obtaining informed consent shall be in accordance of GCP.

(1) Meets the diagnostic criteria for COVID-19 (National Edition 8);
(2) Over 18 years old;
(3) Normal temperature within 48 hours;
(4) With air intake, SpO2>93%;
(5) Onset time >=7 days;

(1) COVID-19 close contacts at centralized isolation sites;
(2) Aged >= 18 years;
(3) Agree to participate in the trial, and the patient or the legal guardian signed the informed consent through paper signature, electronic signature of mobile phone software, or voice authorization.

1. Healthy subjects aged 18-59 years as well as 60 years and above who are capable of providing legal identities. Both males and females should be included;
2. The subjects shall fully understand the content of the informed consent form and the study procedure of this clinical trial. The subjects shall voluntarily sign the informed consent form. The subjects shall be capable of using the thermometer and scale, as well as filling in the Diary.

1. Healthy subjects aged 60 years and above who were capable of providing legal identities. Both males and females should be included;
2. The subjects shall fully understand the content of the informed consent form and the study procedure of this clinical trial. The subjects shall voluntarily sign the informed consent form. The subjects shall be capable of using the thermometer and scale, as well as filling in the Diary.

1. Not yet vaccinated prior to the Screening.
2. Diagnosed with any forms of uveitis in any eye.
3. An inactive disease status (all the following criteria are met):
(1)Anterior chamber cells of 0.5+ or less;
(2)Vitreous haze of 0.5+ or less;

1. Age at the time of the first dose of vaccination: the adult group is 18-59 years old (including the boundary value), the elderly group >=60 years old;
2. Those whose axillary body temperature is less than 37.3 degree C on the day of enrollment;
3. Based on the medical history and relevant physical examination and laboratory examination results, the investigator clinically judges those who are healthy;

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All subjects are required to meet all the following inclusion criteria:
1. Healthy adults aged 18 years and above. Both males and females should be included;
2. The subjects shall fully understand the content of the informed consent form and the study procedure of this clinical trial. The subjects shall be voluntary and capable of signing the informed consent form;

1. Discharged COVID-19 patients;
2. Aged from 18 to 75 years;
3. Those who have fatigue symptom;
4. Have a smart phone and can use it independently or with the help of family members;
5. Volunteer to participate in this study and sign an informed consent form

Inclusion Criteria:

aged 18 to 65 years old (including 18 and 65 years);
male and non-pregnant female;
respiratory tract specimens or hematology samples detected positive results of SARS-CoV-2 by real-

Inclusion Criteria:

At the time of signing ICF, the subjects were 18 to 65 years old (including 18 and 65 years old), both men and women;
Within 72 hours before screening, any samples confirmed by the laboratory were positive for SARS-

(1) Meet NCP critical diagnosis criteria;
(2) Volunteer to participate in this study, cooperate with examination and treatment;
(3) The selected patients were basically stable, conscious and cooperative;
(4) No serious mental disorder;
(5) Aged between 20-80 years

Inclusion Criteria:

Health subjects aged ≥18 years, completed two dose of inactive SARS-CoV-2 vaccine in the past 3-9 months.
The subject can provide with informed consent and sign informed consent form (ICF)

1. Discharged COVID-19 patients;
2. Aged from 18 to 75 years;
3. Those who have fatigue symptom;
4. Have a smart phone and can use it independently or with the help of family members;
5. Volunteer to participate in this study and sign an informed consent form

Inclusion Criteria:

Health subjects aged 60 and above, who have been completed two-dose regimen of inactive SARS-CoV-2 vaccine in the past 3-6 months, or received one dose of inactive SARS-CoV-2 vaccine in the past 1-3 months

Inclusion Criteria:

Healthy adults aged 18-59;
Have been received two doses of inactivated SARS-CoV-2 vaccine(CoronaVac)manufactured by Sinovac Research & Development Co., Ltd at an interval of 21 to 31 days for group A Have been

Inclusion Criteria:

≥18 years old when enrolled;
Participants signing the informed consent;
Being able to participate in all planned follow-ups and accomplish all research procedures including

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Inclusion Criteria:

Severe acute respiratory syndrome coronavirus2 (SARS-CoV-2) infection
Within 7 days from the onset of clinical symptoms or 4 days from the onset of severe symptoms
Consistent with the National Health Committee New Coronavirus pneumonia diagnosis and

Inclusion Criteria:

Adults aged 18-59;
Proven legal identity;
The subjects can understand and voluntarily sign the informed consent form and be willing to

Inclusion Criteria:

Persons aged 18-59 with full capacity for civil conduct;
Subjects voluntarily participate in the study, sign informed consent, provide valid identification, understand and comply with the requirements of the study protocol;

The inclusion criteria of rehabilitation training intervention subjects were: 1. COVID-19 patients; 2. Discharged from hospital; 3. Respiratory dysfunction or motor dysfunction or self-care ability disorder according to relevant functional assessment; 4. Aged 18-70 years; 5. Willing to accept rehabilitation training of this program.

1. Male and female, aged 18-70 years; 2. Novel coronavirus nucleic acid test positive; 3. The diagnostic criteria for confirmed cases in the national COVID-19 Protocol (Eighth edition) were met; 4. Onset within 7 days (severe cases can be extended to less than 10 days); 5. The informed consent is signed by patients or their authorized agent; 6. Subjects who may or receive oral medication; 7. Agree to collect clinical samples; 8. Female subjects of reproductive age were not

1) Persons aged 18 and above; 2) The subject has independent judgment ability, can read, understand and complete the vaccination diary card, and voluntarily participates in it and signs the informed consent; 3) The subject can comply with the requirements of the clinical trial protocol; 4) Armpit temperature <= 37.2 degrees C; 5) No birth plan in the first 3 months after enrollment (including male subjects' partners); Effective contraceptive measures were taken within 2 weeks

1. Patients diagnosed with COVID-19 diagnosed in accordance with the "COVID-19 Diagnosis and Treatment Plan (Trial Eighth Revised Edition)" formulated by the National Health Commission of the People's Republic of China or those who are asymptomatically infected with COVID-19 diagnosed in accordance with the "COVID-19 Prevention and Control Plan (Eighth Edition)";
2. The age of the subject at the time of signing the informed consent form is 18 to 65 years old

1.Aged 18-60 years(including cut-off value), gender not limited;
2.Sputum/nasal swabs/pharyngeal swabs/lower respiratory tract secretions and other specimens, RT-PCR detected the new coronavirus nucleic acid positive, or the viral gene sequencing was highly homologous with the known Delta virus;
3.Patients diagnosed with novel coronavirus meet the diagnostic criteria of the pneumonia diagnosis

All inclusion criteria must be met to qualify for inclusion:
1. The age should be over 18 years old and under 70 years old, with no gender limit. Those who meet the diagnostic criteria of clinical pneumonia
2. According to the clinical diagnosis of viral pneumonia:
(1) Fever (oral temperature >= 38 degrees C, or axillary temperature >= 37.5 degrees C; or fever

Inclusion Criteria:

Healthy subjects aged 18 and above with full capacity for civil conduct, who can provide valid identification;
The subjects voluntarily agree to participate in the study and sign the informed consent to

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1.Individuals aged 18-59 years and are able to provide legal identity certificate. Both male and female should be included.
2.The subjects shall fully understand the content of the informed consent form and the characteristics of the study vaccine, voluntarily sign the informed consent form, as well as have the ability to use thermometer, scale and fill in diary card and contact card as required

Inclusion Criteria:

Children aged 3-5 years;
The subject and/or guardian can understand and voluntarily sign the informed consent form;
Proven legal identity

The subjects eligible for this study must meet all of the following inclusion criteria:
1. Aged >= 18 years, male or female.
2. Body mass index (BMI) of 18-35 kg/m2 (inclusive).
3. The subjects in Cohort A have received two doses of the COVID-19 inactivated vaccine (approved) within 3 months (+15 days) and have been dosed at a time interval of 21 (+10) days

1. Aged 18 to 59 years;
2. Refer to the "New Coronavirus Pneumonia Diagnosis and Treatment Plan (Trial Eighth Edition)" formulated by the National Health Commission, with evidence of etiology or serology, and confirmed mild patients with new coronavirus pneumonia (mild clinical symptoms, no imaging Pneumonia manifestations)

Inclusion Criteria:

Healthy children aged 3-17;
The subjects and/or guardians can understand and voluntarily sign the informed consent form (For subjects aged 8-17 years, both subjects and guardians need to sign the informed consent form)

Inclusion criteria for pulmonary tuberculosis patients:

Pulmonary tuberculosis patients aged 18-75 years who have not received COVID-19 vaccine.
The condition is determined to be stable by the clinician.
The subjects can provide with informed consent and sign informed consent form (ICF)

Inclusion Criteria:

Adults aged 18-45;
Proven legal identity;
The subjects can understand and voluntarily sign the informed consent form and be willing to

Inclusion Criteria:

Healthy participants aged 18 years and above;
Proven legal identity;
The subjects can understand and voluntarily sign the informed consent form and be willing to

Main Inclusion Criteria:

First onset of COVID-19 symptoms <7 days at randomization, symptoms such as fever and/or chills, fatigue, muscle or body aches, headache, new loss of taste or smell, sore throat, congestion or runny nose, nausea or vomiting, and diarrhea

1. Healthy individuals aged 18-59 years as well as 60 years and above who can provide legal identification (males and females are both required).
2. Individuals who are able to understand the contents in the informed consent form and information about the investigational vaccine used in this study; willing to sign the informed consent form; able to use the thermometer, scale as well as fill in the diary card and contact card as required

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Inclusion Criteria:

Male or female healthy subjects
Body weight ≥ 50 kg for males and ≥ 45 kg for females
Female subjects must meet the following criteria: not of childbearing potential (e.g., documented

Inclusion Criteria:

18 ≤age ≤59 years;
Vaccinated with two doses of SARS-CoV-2 vaccine (manufactured by Sino Biopharmaceutical Limited or SINOVAC BIOTECH CO., LTD.) (≥6 months from the time of vaccination)

Inclusion Criteria:

18 ≤age ≤59 years;
Vaccinated with two doses of SARS-CoV-2 vaccine (manufactured by Sino Biopharmaceutical Limited or SINOVAC BIOTECH CO., LTD.) (≥6 months from the time of vaccination)
1. meet the new coronary virus pneumonia recovery period diagnostic criteria;
2. short breath, sweating, dry cough, chest tightness, palpitations, etc., three symptoms, or single symptom visual simulation score method (VAS) >=4;
3. the cure discharge is more than 2 weeks;
4. aged 18 ~ 70 years old;

1) Subjects aged 18 years and older;
2) Subjects capable of understanding and voluntarily signing the informed consent;
3) Subjects with general good health as judged by medical history and physical examination. General good health was defined as no significant change of treatment or hospitalization due to disease deterioration within 3 months prior to enrollment;

1.Adult subjects (18 years old), no gender limitation;
2.Subjects in good health or stable clinical situation;
3.Subjects who completed two doses of inactivated COVID-19 vaccine 4-8 months ago;
4.Female subjects of childbearing age are not breast-feeding or pregnant (negative blood pregnancy test), and have no family planning in the first 3 months after enrollment. Take effective contraceptive

1. Individuals aged 18 years and above and are able to provide legal identity certificate. Both male and female should be included;
2 .The subjects shall fully understand the content of the informed consent form and the characteristics of the study vaccine, voluntarily sign the informed consent form, as well as have the ability to use thermometer, scale and fill in diary card and contact card as required;

(1) meet the critical diagnosis criteria for severe COVID-19, (2) are aged between 20 and 80?years and are male or female, (3) have a stable condition and are conscious and cooperative in the examination, (4) volunteer to join the trial and sign the informed consent form, and (5) promise not to perform other exercise programs.

1. Subjects who have received 2 doses of pGX9501 (1.0 mg/2.0 mg) 12 months (+/-2 months) ago;
2. Body mass index (BMI) is between 18-35 kg/m2 (including upper and lower limits);
3. Axillary temperature < 37.3 ℃ on the day of vaccination;
4. Subjects are considered by the investigator as healthy subjects eligible for immunization with this product based on inquiry of medical history and inspection of the physical examination results;

Inclusion Criteria:

Health subjects aged ≥18 years.
Have received two-dose inactivated SARS-CoV-2 vaccine before 6 months or more.
The subject can provide with informed consent and sign informed consent form (ICF)

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Inclusion Criteria:

Male or female adults who are between 18 and 60 years old inclusive
Weigh at least 45kg, with a BMI of 19 to 30 kg/m2 inclusive
No serious underlying disease which would adversely affect the study conduct and data

Inclusion Criteria:

Age ≥ 18 years old,
Through asking medical history and physical examination, the investigator judged that the health condition is good; the chronic disease is stable and there is no change within 3 months of

Inclusion Criteria:

Aged ≥18 years old when signing ICF;
No other anti-SARS-COV-2 vaccines (approved for marketing or registered study) have been previously administered;

Inclusion Criteria:

Adults ≥ 18 years
Negative results of severe acute respiratory syndrome coronavirus 2 (SARS-CoV-2) quantitative reverse transcriptase polymerase chain reaction (qRT-PCR)

① Patients with confirmed mild or common form of novel coronary pneumonia infection. ② Age ≥ 18 years. ③ Clear consciousness, with the ability to express themselves verbally. ④ Those who breathe spontaneously and can cooperate with this experiment.

(1) Inpatients with COVID-19; (2) Those who have received similar antibiotic treatment; (3) Patients who can get out of bed; (4) Individuals who show interest are invited to conduct face-to-face interviews to screen research qualifications.

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Exclusion Criteria

Exclusion criteria: 1. Any situation that makes the programme cannot proceed safely;
2. Known allergy or hypersensitivity reaction to lopinavir / ritonavir;
3. Increase in alanine aminotransferase (ALT) / aspartate aminotransferase (AST) is more than 5 times the upper limit of normal;
4. Use of medications that are contraindicated with lopinavir / ritonavir

Exclusion Criteria:

pregnancy;
patients currently taking corticosteroids (cumulative 400 mg prednisone or equivalent);

Exclusion Criteria:

Age < 14 years or > 80 years;
Pregnant or lactating female;
One of the following items occurred at the enrollment: (i) respiratory failure

Exclusion criteria: 1. Patients are allergic or intolerant to therapeutic drugs;
2. Pregnancy or lactating women;
3. Researchers believe that patients may have other factors affecting the efficacy or safety evaluation of this study.

Exclusion criteria: (1) Patients are diagnosed as a case of severe NCIP;
(2) Patients are pregnant or breastfeeding women;
(3) Patients have aspartate aminotransferase (AST) or alanine aminotransferase (ALT) > 5Ã—UNL or creatinine clearance <50 ml / min;
(4) Patients are allergic or intolerant to the proposed antiviral therapeutic drugs;
(5) Patients are

Exclusion Criteria:

Hypersensitivity to darunavir, cobicistat, or any excipients
Patients with severe liver injury (Child-Pugh Class C)
Concomitant medications that are highly dependent on CYP3A clearance

Exclusion criteria: 1) Severe basic diseases that affect survival, including: uncontrolled malignant tumors with multiple metastases that cannot be removed, hematological diseases, cachexia, active bleeding, severe malnutrition, HIV, etc.;
2) Obstructive pneumonia caused by lung tumors, severe pulmonary interstitial fibrosis, alveolar proteinosis, allergic

Exclusion Criteria:

Disturbance of consciousness
Difficulty swallowing and frequent vomiting
Patients requiring blood transfusion

Exclusion Criteria:

Have a clear history of lopinavir or ritonavir or arbidol allergy
Severe nausea, vomiting, diarrhea and other clinical manifestations affect the oral or absorption of the drugs

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Exclusion Criteria:

Physician makes a decision that trial involvement is not in patients' best interest, or any condition that does not allow the protocol to be followed safely

Exclusion Criteria:

Physician makes a decision that trial involvement is not in patients' best interest, or any condition that does not allow the protocol to be followed safely

Exclusion criteria: 1. Those with allergies;
2. Patients with a history of severe cardio-cerebral vascular disease, diabetes, mental and neurological diseases, severe liver and renal insufficiency;
3. History of hip or knee trauma or surgery;
4. Patients who abuse drugs;
5. Those who are in the acute phase of chronic diseases;
6. Patients with other malignant

Exclusion criteria: 1. Respiratory tract infection caused by primary immunodeficiency disease, acquired immunodeficiency syndrome, congenital respiratory tract malformation, congenital heart disease, gastroesophageal reflux disease, abnormal lung development and other basic diseases, with clear evidence of bacterial infection;
2. Subjects with the following

1. There is clear evidence of bacterial infection in respiratory tract infections caused by basic diseases such as primary immunodeficiency disease, acquired immunodeficiency syndrome, congenital respiratory malformations, congenital heart disease, gastroesophageal reflux disease, and abnormal lung development;

Exclusion criteria: 1. Pregnancy or lactation;
2. Previous serious chronic basic diseases;
3. Previous allergy to traditional Chinese medicine;
4. Newly diagnosed 2019-nCOV pneumonia.

Exclusion criteria: 1) patients were judged not suitable to participate in the study;
2) patients were not suitable to use Chinese patent medicine.

Exclusion criteria: 1) Pregnant or lactating women;
2) Severe basic diseases that affect survival;
3) Other problems thant doctor's judgment is not suitable for the study;
4) Patients have difficulty in taking TCM standard decoctions.

Exclusion criteria: 1. Patients with serious illnesses, such as heart, liver, kidney, endocrine diseases and hematopoietic system disease;
2. Pregnant or lactating women;
3. Patients with mental confusion, with a history of drug abuse or dependence, who allergic to study medication, who have participated in another clinical trial within 3 months or who have other

Exclusion criteria: 1. Patients with serious illnesses, such as heart, liver, kidney, endocrine diseases and hematopoietic system disease;
2. Pregnant or lactating women;
3. Patients who have mental confusion, who with a history of drug abuse or dependence, who allergic to study medication, who have participated in another clinical trial within 3 months or

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Exclusion criteria: 1. Patients with serious illnesses, such as heart, liver, kidney, endocrine diseases and hematopoietic system disease;
2. Pregnant or lactating women;
3. Patients who have mental confusion, who with a history of drug abuse or dependence, who allergic to study medication, who have participated in another clinical trial within 3 months or

Exclusion criteria: 1. Patients with serious illnesses, such as heart, liver, kidney, endocrine diseases and hematopoietic system disease;
2. Pregnant or lactating women;
3. Patients who have mental confusion, who with a history of drug abuse or dependence, who allergic to study medication, who have participated in another clinical trial within 3 months or

Exclusion criteria: 1. Patients with serious illnesses, such as heart, liver, kidney, endocrine diseases and hematopoietic system disease;
2. Pregnant or lactating women;
3. Patients who have mental confusion, who with a history of drug abuse or dependence, who allergic to study medication, who have participated in another clinical trial within 3 months or

Exclusion Criteria:

Patients who meet any of the contraindications in the experimental drug labeling

Patients who do not want to participate in this clinical study

Exclusion Criteria:

Patients who meet any of the contraindications in the experimental drug labeling

Patients who do not want to participate in this clinical study

Exclusion Criteria:

dying state (i.e. survival time is less than 24 hours); progressive malignant tumor with life expectancy less than 6 months; immunosuppressive status(patients using immunosuppressants or

Exclusion criteria: (1) any situation in which the program cannot proceed safely;
(2) allergic constitution;
(3) serious basic diseases affecting survival, including uncontrolled malignant tumors with multiple metastases that cannot be resected, blood diseases, malignant fluid, active bleeding, severe malnutrition, HIV, etc ;
The main organ function is abnormal

Exclusion criteria: (1) Patients are not suitable to take Chinese medicine decoction.
(2) Patients have other serious organ disease or mental illness;
(3) Patients allergy or have contraindications to the drugs involved in the research program.

Exclusion criteria: (1) The critically ill patients in the diagnosis and treatment program of pneumonia with new coronavirus infection issued by the National Health Commission;
(2) It is not suitable for patients who take traditional Chinese medicine decoction;
(3) Those with other serious organ diseases or mental diseases;
(4) Those who are allergic to or have

Exclusion criteria: 1) Any situation that makes the programme cannot proceed safely;
2) patients who have used Kaletra or remdesivir;
3) no clinical or chest imaging manifestations;
4) In the state of no oxygen at rest, the patient's SPO2 <= 94% or the oxygenation index is less than 300mmHg respiratory rate >= 24/min;
5) Patients who need to be

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Exclusion criteria: 1. Allergic history to study medicines;
2. ALT/AST >5 UNL or Child-Pugh Class C;
3. Severely ill patients with life expectance <48 hours;
4. Contraindication of DRV or thymosin;
5. Pregnancy testing positive for child-bearing woman;
6. Known HIV infection;
7. not adequate to be enrolled

Exclusion criteria: 1. Allergic constitution, known to be allergic to baiosavir or farpiravir or pharmaceutical excipients;
2. Body weight <40 kg;
3. Considered as severe disease: has an ongoing respiratory deficiency and subjected to invasive mechanical ventilation; or presence of shock; or admitted to the ICU due to complications;
4. Has known kidney

Exclusion criteria: 1. Hypersensitive to study drug;
2. Body weight <40 kg;
3. Considered as severe disease: has an ongoing respiratory deficiency and subjected to invasive mechanical ventilation; or presence of shock; or admitted to the ICU due to complications;
4. Has known kidney dysfunction determined as CLcr<60 ml /min;
5. Increase in alanine

Exclusion criteria: Exclude patients with severe and critical 2019-nCoV pneumonia

Exclusion criteria: The investigator considers that the subject has other conditions that make him/her unsuitable to participate in the clinical trial or other special circumstances.

Exclusion criteria: (1) diseases need to be differentiated from 2019-ncov, such as influenza virus, parainfluenza virus, adenovirus, respiratory syncytial virus, rhinovirus, human partial pulmonary virus, SARS coronavirus and other known viral pneumonia, mycoplasma pneumoniae, chlamydia pneumonia, bacterial pneumonia and non-infectious diseases;
(2) suspected case of

Exclusion Criteria:

1.2019-nCoV severe Pneumonia patients.

Meet the definition of severe pneumonia(Comply with any of the followings):

Exclusion Criteria:

Allergic to vitamin C;
Dyspnea due to cardiogenic pulmonary edema;
Pregnant or breastfeeding;

Exclusion criteria: 1. Have any known disease that seriously affects the immune system, such as a history of infection with the human immunodeficiency virus (HIV), or malignant tumors of the blood system or solid organs, or splenectomy;
2. Other circumstances that the researcher considers inappropriate to participate in this study

Exclusion criteria: You cannot be enrolled if you meet any of the following criteria:
1) any situation that cannot allow the program to proceed safely;
2) allergic constitution, allergic to more than one kind of food or medicine;
3) patients with severe basic diseases that affect survival, including uncontrolled clinically significant heart, lung, kidney, digestion

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Exclusion criteria: 1. Patients with other malignant tumors requiring treatment;
2. Severe cardiovascular and metabolic diseases beyond the control of combination with other drugs
3. Patients with clinical symptoms of brain dysfunction or serious mental illness who cannot understand or follow the study protocol;
4. Endotracheal intubation with

Exclusion criteria: (1) Immunodeficiency diseases, or those using immunosuppressive agents or glucocorticoids within the past 3 months;
(2) Pregnancies, pregnant women and lactating women;
(3) An allergic condition, such as a history of allergies to two or more drugs or foods, or a known allergy to the ingredients in this test);
(4) Psychotic

Exclusion criteria: 1. Clear evidences?of bacterial infection;
2. Serious primary diseases judged by researchers,such as heart, kidney, lung, endocrine, blood, metabolism and gastrointestinal tract,?may affect the patients' participation in the trial or the outcome of the study;
3. Family history of mental illness or mental illness;
4. Allergic constitution or multi

Exclusion criteria: 1. Severe or critically severe patients;
2. Clear evidence of bacterial infection;
3. Patients with severe primary diseases such as heart, kidney, lung, endocrine, blood, metabolism, and gastrointestinal tract, etc., may affect participation in the trial or may affect the outcome of the study by the judgment of the researcher;
4. Patients

Exclusion criteria: 1. Patients with severe 2019-nCoV pneumonia met one of the following conditions: respiratory distress, RR >= 30 times / min; or SaO2 / SpO2 <= 93% in resting state; or arterial partial pressure of oxygen (PaO2) / concentration of oxygen (FiO2) <= 300MMHG (1mmhg = 0.133kpa);
2. Patients with critical 2019-nCoV pneumonia met one of the following

Exclusion criteria: 1. Severe pneumonia requires mechanical ventilationcritically severe cases;
2. Estimated Time of Death is less than 48 hours;
3. There is clear evidence of bacterial infection in respiratory tract infections caused by basic diseases such as primary immunodeficiency disease, acquired immunodeficiency syndrome, congenital

Exclusion Criteria:

Hypersensitivity to chloroquine or hydroxychloroquine;
Women during pregnancy;
Severe heart, lung, kidney, brain, blood diseases or other important systemic

Exclusion criteria: 1. Pregnant or lactating women;
2. There are comorbidities that affect the judgment of the efficacy, such as those with malignant tumors or long-term immunosuppressants;
3. The investigator believes that the patient has other conditions that are not suitable for enrollment;
4. Allergic to dimethyl sulfoxide (DMSO), dextran 40 or

Exclusion criteria: 1. Pregnant or lactating women;
2. Patients who cannot be inhaled by atomization;
3. Patients who are intolerant or allergic to interferon preparations;
4. Other patients were judged unsuitable for inclusion in the trial by researchers.

Exclusion criteria: 1. Known to receive hormone therapy orally or intravenously;
2. Hormone therapy is needed due to concomitant disease upon admission;
3. Patients with diabetes are receiving oral medication or insulin therapy;
4. Known contraindications to dexamethasone or other excipients (such as refractory hypertension;

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Exclusion criteria: 1. Intubation required immidiately in case of cardiac arrest or asphyxia;
2. Pregnancy, severe chronic respiratory, uncontrolled gastric reflux disease and any nasopharyngeal anatomical obstacle;
3. Those patients has intubated in ICU.

Exclusion criteria: Patients who is enrolled in other clinical trial
who had diagnosed as macular degeneration of retinawere or defects of visual field;
who was allergic to 4-aminoquinoline compounds or is prescribed any drug had interactions with hydrocholoroquine
emergency or critically ill patients who can not received oral drugs
pregnancy or lactation

Exclusion criteria: (Subjects cannot enter the study if they meet any of the following conditions):
1. Patients with a history of allergy to chloroquine phosphate, lopinavir, and ritonavir;
2. Patients with hematological diseases;
3. Patients with chronic liver and kidney disease and reaching the end stage;
4. Patients with arrhythmia and chronic heart disease;

Exclusion criteria: 1. Pregnant or lactating women, or pregnant;
2. Those who are allergic to the drug ingredients of this program;
3. Severe cardiovascular diseases, such as severe center of gravity dysfunction, second or third degree atrioventricular block, etc.;
4. Severe hepatic insufficiency: defined as the upper limit of ALT > 2 times normal value or AST

Exclusion criteria: Patients over 80 years of age or less than 12 years of age (trial 3 and less than 18 years old); patients with contraindications to study drugs; cancer patients, life expectancy <6 months; participants in other clinical trials in the past three months; pregnant women, breastfeeding Women

Exclusion criteria: 1. Patients with severe pneumonia requiring mechanical ventilation and severe new coronavirus infection with pneumonia;
2. Death is expected within 48 hours;
3. There is clear evidence of bacterial infection in respiratory tract infections caused by primary immunodeficiency disease, acquired immunodeficiency syndrome, congenital

Exclusion criteria: 1. Chronic lung diseases requiring long-term oxygen therapy;
2. Long-term application of glucocorticoids due to underlying diseases;
3. Based on the researcher's judgment, previous or current diseases may affect patients' participation in the trial or influence the outcome of the study, including: malignant disease, autoimmune disease

Exclusion criteria: 1. The doctor believes that the patient is not suitable to participate in this trial, including those who may not cooperate, do not comply with the requirements of the procedure, or participate in this trial may put the patient in an unsafe situation.
2.Pregnant or lactation periods women;
3.Immunoglobulin allergy;
4.Immunoglobulin A deficiency;

Exclusion criteria: 1. Severe liver disease (such as child Pugh score >= C, AST > 5 times upper limit);
2. The patients were allergic to the components in asc09 / ritonavir compound tablets;
3. Patients with contraindications specified in the instructions of ritonavir tablets;
4. The pregnancy test of female subjects in the screening period was positive;

Exclusion criteria: 1. Known allergic to hydroxychloroquine or 4-aminoquinoline;
2. Severe renal impairment (eGFR <= 30 mL/min/1.73 m2), and those with RRT;
3. Severe liver disease (Child Pugh score >= C, AST>5 times upper limit);
4. Pre-existing retinopathy of the eye;
5. History of epilepsy;
6. G6PD deficiency;
7. Pregnant or

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Exclusion criteria: Patients with any of the following were excluded from the study:
1. Patients with previous allergy to 4-aminoquinoline compounds;
2. Patients with previous severe retinal or visual field related diseases;
3. Patients with severe liver dysfunction and patients with renal dysfunction (creatinine clearance rate <= 50 mL / min);
4. Patients

Exclusion criteria: Patients with any of the following were excluded from the study:
1. Patients with previous allergy to 4-aminoquinoline compounds;
2. Patients with previous severe retinal or visual field related diseases;
3. Patients with severe liver damage;
4. Patients with impaired renal function (creatinine clearance <=50 mL / min);
5. Patients weighing

Exclusion criteria: (1) Patients who can not guarantee compliance of using TCM during the treatment period, or patients who are difficult to take medicine by oral or nasal route.
(2) Patients who have been more than 7 days since definite diagnosis.
(3) Patients with severe primary respiratory disease or other pathogenic microbial pneumonia that needs to

Exclusion criteria: 1. Patients who are participating in other drug clinical trials;
2. Pregnant or lactating women;
3. ALT / AST> 5 ULN, neutrophils <0.5, platelets less than 50;
4. Definite diagnosis of rheumatic immune-related diseases;
5. Long-term oral anti-rejection or immunomodulatory drugs;
6. Hypersensitivity to tocilizumab or any excipients;
7.

Exclusion criteria: 1. Infected with other viruses such as HCV, HIV, and syphilis;
2. Used non-steroidal anti-inflammatory drugs within 3 days before enrollment;
3. Have severe liver disease;
4. Those who are known to be allergic to the test drug and its ingredients;
5. Any of the following risk factors:
1) Chronic respiratory diseases such as chronic

Exclusion criteria: Children, pregnant women

Exclusion criteria: 1. Severe pneumonia requires mechanical ventilationcritically severe cases;
2. Estimated Time of Death is less than 48 hours;
3. There is clear evidence of bacterial infection in respiratory tract infections caused by basic diseases such as primary immunodeficiency disease, acquired immunodeficiency syndrome, congenital

Exclusion criteria: 1. Pregancy or lactating women;
2. People with allergies or allergies to components of Truncated Torsion formula;
3. Patients with serious basic diseases such as malignant tumor, cirrhosis, chronic renal failure (uremic stage), hematological disease, HIV, etc.;
4. Patients with obstructive pneumonia caused by lung tumor, pulmonary

Exclusion criteria: 1)One of the following conditions:
Respiratory distress, RR >= 30 times/min;
At rest, oxygen saturation is less than 93%;
Artial arterial oxygen pressure (PaO2)/oxygen absorption concentration (FiO2) <= 300mmHg (1mmHg=0.133kPa);
Respiratory failure is present and mechanical ventilation is required;
Shock;

Exclusion criteria: 1)One of the following conditions:
Respiratory distress, RR >= 30 times/min;
At rest, oxygen saturation is less than 93%;
Atrial arterial oxygen pressure (PaO2)/oxygen absorption concentration (FiO2) <= 300mmHg (1mmHg=0.133kPa);
Respiratory failure is present and mechanical ventilation is required;
Shock;

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Exclusion criteria: 1. People with allergies and allergies to experimental drugs;
2. Combined with other malignant tumors;
3. Patients with hematological diseases, coagulopathy and autoimmune diseases;
4. Combined with severe cardiovascular diseases, Patients with vascular disease, hematopoietic system disease, and neuropathy;
5. Participated

Exclusion criteria: 1. Patients with cognitive disorder or known mental illness;
2. Pregnant or lactating women;
3. Patients with active tuberculosis, pulmonary abscess or chronic respiratory failure;
4. Patients with tumor;
5. Patients with severe liver or kidney diseases;
6. Patients with severe cardiovascular or cerebrovascular diseases;
7. Patients who are

Exclusion criteria: (1) Impaired consciousness or mental illness cannot cooperate with treating the observer.
(2) Those with severe organ dysfunction.
(3) Pregnant women.

Exclusion Criteria:

1. Pregnancy or breastfeeding
2. Known HIV, HBV or HCV infection
3. Patients with malignant tumor, other serious systemic diseases and

Exclusion criteria: 1. The physician decides that participating in the trial is not in the best interest of the patient, or does not meet any conditions for safe follow-up in the protocol;
2. People with heart disease such as cardiac block;
3. People with eye diseases such as retina and cornea;
4. Severe liver disease (such as Child Pugh score >= C or AST > 5 times the

Exclusion criteria: 1. Aged <18 years;
2. Pregnant or lactating women;
3. Allergic to the test drug;
4. The underlying disease is very serious, and the expected survival time is less than 6 months (such as advanced malignant tumors);
5. COPD or end-stage lung disease requires home oxygen therapy;
6. Expected survival time does not

Exclusion criteria: (1) Immunodeficiency disease, or those who have used immunosuppressive agents or glucocorticoids within the past 3 months;
(2) Preparing pregnant, pregnant and lactating women;
(3) Patients with allergies (referring to allergies to two or more medicines, foods or known ingredients in this study);
(4) Patients with mental illness, or have no

Exclusion criteria: (1) Those who cannot take Chinese traditional medicine decoction;
(2) mentally ill subjects who are not easy to control;
(3) those who are pregnant or breast-feeding;
(4) those who use other Chinese medicines;
(5) those who are not considered suitable to participate in this trial by researchers.

Exclusion criteria: If subject meet one of the following exclusion criteria, he/she cannot be enrolled in this study:
1. Be dysphagia and cannot receive drud by oral administration;
2. Complicated with pneumonia infected by other virus;
3. Allergy to or intolerance to study drug phosphoric chloroquine;
4. With significantly abnormal liver function(

Exclusion Criteria:

Patients with severe allergies or allergies to stem cell preparations and their components;

Patients with serious basic diseases that affect survival, including: blood

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Exclusion Criteria:

ALT≥5 times of ULN, level of total bilirubin≥3 times of ULN, or Cr≥1.5 times of ULN.

Patients with serious severe liver disease

Exclusion criteria: 1. Serious/critically ill patients with novel coronavirus(COVID-19) pneumonia or patients that clinician consider that he/she shall be treated in ICU;
2. With pneumonia infected by other virus;
3. With one of following abnormal laboratory parameters(judged per local Laboratory reference range):
-ALT or AST≥5 ULN or;
-ALT

Exclusion criteria: Participants who have used fluoroquinolone antibiotics within 1 week

Exclusion criteria: 1. Involving in other drug clinical trial;
2. Pregnant or breastfeeding;
3. Presence of other serious diseases that may be life threatening, such as cancer;
4. Expected life is less than 24 hours;
5. Allergy to ALA or drugs of similar chemical classes (Vitamin B) previous history of intolerance to recommended target doses of alpha-LA;

Exclusion criteria: (1) known or suspected allergy to the components of Azivudine tablets;
(2) patients with malabsorption syndrome or any other condition that affects gastrointestinal absorption and requires intravenous nutrition or unable to take oral medication;
(3) patients who is currently receiving anti-hiv treatment;
(4) patients with one of the following

Exclusion criteria: 1) pneumonia patients with severe or critical new coronavirus infection;
2) daily treatment of asthma, any other chronic respiratory disease;
3) patients with confirmed history of diabetes and HbA1C >=8.0%;
4) those allergic to the known ingredients of the drug;
5) pregnant or nursing women;
6) patients with severe mental

Exclusion criteria: 1. Patients who have received immunotherapy (such as PD-1 / L1, CTLA4, etc.) and inflammatory factor modulators (such as Ulinastatin) within one month.
2. Patients who have used antibacterial drugs such as macrolides within one week.
3. Patients who have received other anti-2019-nCOV potentially effective drugs such as Lopinavir

Exclusion criteria: 1. Aged less than 18 years old;
2. Other serious medical diseases such as malignant tumor, heart, liver and kidney disease, uncontrollable metabolic disease, etc;
3. Not suitable for gastrointestinal administration;
4. Pregnant or lactating women;
5. Those who are allergic to the ingredients of this product;
6. Mental state can not

Exclusion criteria: 1. Pregancy or lactating women;
2. People with allergies or allergies to traditional Chinese medicine
3. Severe Primary disease such as uncontrolled and metastasis malignancy, hematopathy, HIV;
4. Obstructive pneumonia caused by lung tumors, severe pulmonary interstitial fibrosis, alveolar proteinosis, allergic reaction alveolitis;
5

Exclusion Criteria:

severe immunosuppression (HIV infection, long-term use of immunosuppressive agents

pregnant or lactation period women

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Exclusion criteria

Patients who have undergone lung surgery that affects pulmonary function, such as pulmonary transplantation, pulmonary resection, pulmonary volume reduction, etc;

Exclusion criteria: 1 Patients with identified allergies to 4-aminoquinoline and lopinavir/litonavir;
2 Patients with server diseases of the blood system;
3 Patients with identified severe liver disease, underlying cirrhosis or alanine aminotransferase (ALT)/aspartate aminotransferase (AST) elevation more than 3 times the normal upper limit;
4 Patients with alcoholism or

Exclusion criteria: 1. Patients with identified allergies to 4-aminoquinoline and lopinavir/litonavir;
2. Patients with server diseases of the blood system;
3. Patients with identified severe liver disease, underlying cirrhosis or alanine aminotransferase (ALT)/aspartate aminotransferase (AST) elevation more than 3 times the normal upper limit;
4. Patients with alcoholism or

Exclusion criteria: 1. Female patients during pregnancy;
2. Patients with known allergy to chloroquine;
3. Patients with haematological diseases;
4. Patients with chronic liver or kidney diseases at the end stage;
5. Patients with arrhythmia and chronic heart disease;
6. Patients with retinal disease, hearing deficiency or hearing loss;
7. Patients with

Exclusion criteria: 1. Patients with respiratory failure, acute necrotizing encephalopathy, septic shock, multiple organ dysfunction, and other serious clinical conditions requiring monitoring and treatment when enrolled;
2. Pregnant, preparing or lactating women;
4. Patients with severe heart, cerebrovascular and liver, kidney, hematopoietic system diseases and

Exclusion criteria: 1. Patients with respiratory failure, acute necrotizing encephalopathy, septic shock, multiple organ dysfunction, and other serious clinical conditions requiring monitoring and treatment when enrolled;
2. Pregnant, preparing or lactating women;
3. Patients with severe heart, cerebrovascular and liver, kidney, hematopoietic system diseases and

Exclusion criteria: 1. There is clear evidence of bacterial infection in respiratory tract infection caused by basic diseases such as primary immunodeficiency disease, congenital respiratory tract malformation, congenital heart disease, gastroesophageal reflux disease and pulmonary dysplasia;
2. Subjects with the following conditions: asthma requiring

Exclusion criteria: No

(1) Positive for other pathogens such as A, B, Mycoplasma, Adenovirus;
(2) patients require mechanical ventilation;
(3) People with metal or pacemaker;
(4) active tuberculosis;
(5) Massive pulmonary fibrosis;

Exclusion criteria: 1. The serious NCP patients who can neither swallow medicines nor take medicine via stomach tube, and the critical serious NCP patients;
2. Those with severe hypo-immunity due to, but not limited to, one of the following reasons:
1) HIV/AIDS, the condition is still unstable in the process of mediation and /or the last detection value of CD4 plus T

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Exclusion criteria: 1. Patients with other serious diseases such as lung diseases, cardiovascular and cerebrovascular diseases, hematopoietic diseases, autoimmune diseases, digestive system, or mental illness;
2. Pregnant or lactating women;
3. Participate in other forms of exercise during the trial

Exclusion criteria: 1) Cases with severe vomiting and difficulty in taking drugs orally or causing difficulty in taking drugs after oral administration;
2) Pregnant and lactating women;
3) The subjects received specific antiviral drugs such as lopinavir / lidonavir, ribavirin and monoclonal antibody within one week before admission;
4) Cases with no hope of survival

Exclusion criteria: 1. critically ill patients based on the New Protocol for Diagnosis and Treatment of NCP;
2. Patients with eye diseases (such as cataract, conjunctivitis, glaucoma)
3. Severe liver disease (such as AST > 3 ULN);
4. Patients with severe renal insufficiency (estimated glomerular filtration rate <=30ml/min/1.73m2) or continuous renal

Exclusion criteria: 1. Malignant disease in the past five years;
2. The clinical prediction is that there is no hope for survival, only cases of hospice care, or cases of deep coma that do not respond to supportive treatment measures within three hours of admission;
3. Subjects who are participating in other clinical trials or within 3 months;
4. Cases of very

Exclusion criteria: 1. Expected to die within 48 hours;
2. Patients with asthma attack, purulent tonsillitis, acute and chronic bronchitis, sinusitis, otitis media, and other respiratory diseases affecting clinical trial evaluation; and chest CT confirmed the existence of severe pulmonary interstitial disease, bronchiectasis, obstructive pulmonary disease With lung disease;
3.

Exclusion criteria: 1. In the opinion of the researchers, the participation of the trial is not in the best interests of the patients, or any situation is not in line with the situation that the trial scheme is allowed to be followed safely;
2. According to the judgment of the investigator, the diseases previously or now may affect the patients' participation in the trial or the outcome of the study

Exclusion criteria: 1. Critical case with the following situation:
 i. Patients with respiratory failure and needing mechanical ventilation;
 ii. Patients with shock;
 iii. Patients with other organ failures and needing ICU monitoring and treatment;
2. Patients who have other respiratory tract infection;
3. Patients used Liu Shen Capsule within 1 month before

Exclusion criteria: 1. Patients with other serious diseases such as lung diseases, cardiovascular and cerebrovascular diseases, hematopoietic diseases, autoimmune diseases, digestive system, or mental illness;
2. Pregnant or lactating women;
3. Participate in other forms of exercise during the trial

Exclusion criteria: (1) The PI considers that the patient is not suitable to participate the study or the patient may be unsafe if he participate the study;
(2) Patients with serious liver disease, grade C according to child-pugh;
(3) Patients with severe renal impairment (glomerular filtration rate <=30ml /min(1.73m2) or who have received continuous renal replacement

Exclusion criteria: 1. Patients who are participating in other drug clinical trials;
2. Pregnant or lactating women;
3. ALT / AST> 5 ULN, neutrophils <0.5, platelets less than 50;
4. Expected life<1 week;
5. Definite diagnosis of asthma-related diseases;
6. Long-term oral anti-rejection or immunomodulatory drugs;
7. Hypersensitivity to tocilizumab or any

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Exclusion criteria: No

Exclusion criteria: (1) Hyperthyroidism;
(2) Favism;
(3) Pregnancy;
(4) Abnormal coagulation, thrombocytopenia, active bleeding;
(5) People with medical ozone allergy or hypersensitivity;
(6) Severe anemia;
(7) Hypocalcemia;
(8) Unstable period of severe cardiovascular disease;
(9) Uncontrolled diabetes

Exclusion criteria: 1. Combining basic diseases such as hypertension, diabetes, and coronary heart disease;
2. Malignant tumor or with a history of malignant tumor;
3. pregnant or lactating women;
4. suffer from severe mental illness;
5. History of unstable angina pectoris or myocardial infarction within 6 months; coronary angiography suggested

Exclusion criteria: 1. The clinical classification of patients with severe novel coronavirus infection is to meet any of the following:
1) Respiratory failure occurs and requires mechanical ventilation;
2) Shock occurs;
3) Combined failure of other organs requires ICU monitoring and treatment;
2. Those who are allergic to blood products or plasma

Exclusion criteria: 1. Pregnant women;
2. Patients with a history of angina pectoris, myocardial infarction and other serious cardiovascular diseases;
3. The researchers judged that this study was not in the interests of patients.

Exclusion Criteria:

Allergies, known to be allergic to research drugs or drug excipients;
Patient weight is less than 40 kg;
Respiratory distress-RR>30 / min on room air or SPO2< 93%, or PaO2/FiO2

Exclusion criteria: 1. Females who are pregnant or likely to be pregnant, or who are lactating and unable to stop breastfeeding, or who have positive pregnancy tests during screening;
2. Males or females who have a birth plan or are unwilling to take reliable contraceptive measures for contraception within 90 days from signing the informed consent to the last

Exclusion criteria: Exclude patients with influenza, adenovirus and other upper respiratory tract infections

Exclusion criteria: (1) Patients with known or suspected to be allergic to the components of the test drug;
(2) Patients with known hepatitis B, C, AIDS and other viral infections;
(3) Patients with severe or severe clinical classification who have experienced complications such as respiratory failure and acute respiratory distress syndrome (ARDS);
(4)

Exclusion criteria: (1) Subjects who have any active autoimmune disease or history of autoimmune disease;
(2) Subjects who received immunosuppressive therapy by using immunosuppressant, or systemic or absorbable topic hormone therapy (temprednisone or other therapeutic hormones with a dosage ≥10 mg/day) within 14 days prior to the

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Exclusion criteria: 1. Serious/critically ill patients with novel coronavirus(COVID-19) pneumonia or patients that clinician consider that he/she shall be treated in ICU;
2. With pneumonia infected by other virus;
3. With one of following abnormal laboratory parameters(judged per local Laboratory reference range):
-ALT or AST>5 ULN or:
-ALT

Exclusion criteria: 1. The physician decides that participating in this study is not in the best interest of the subject or that there exist a circumstance in which the agreed protocol cannot be safely followed;
2. People who have allergic reactions or allergies to antiviral oral solution or Wuzhifangguanfang decoction;
3. Those who do not meet the national

Exclusion criteria: (1) Diagnosed with other known viral pneumonia,like influenza virus, parainfluenza virus, adenovirus, respiratory syncytical virus, rhinovirus, human metapneumovirus and SARS coronavirus, etc.;
(2) Diagnosed with mycoplasma pneumonia, chlamydial pneumonia and bacterial pneumonia;
(3) Diagnosed with vasculitis, dermatomyositis

Exclusion criteria: 1. Pregnancy or lactating women;
2. People with allergies or allergies to Shenfu injection and its components;
3. Severe basic diseases that affect survival, including: uncontrolled malignant tumors with multiple metastases that cannot be removed, hematological diseases, cachexia, active bleeding, severe malnutrition, HIV, etc.;
4. Obstructive

Exclusion Criteria:

Age <18 years; Age >80 years
Pregnant or breast feeding woman or with positive pregnancy test result
P/F <100 mmHg

Exclusion criteria: 1. Investigator makes a decision that trial participation is not in patients' best interest, or any condition that does not allow the protocol to be followed safely;
2. As judged by investigator historic or current diseases that may have impact on the subjects participation of the study or the study outcome. These diseases include but not limited to cadiac diease

Exclusion criteria: 1. Patients with presence, coagulopathy or hypotension;
2. Pregnant and lactating women;
3. People with allergies or allergies to Xuebijing Injection and its components;
4. Severe basic diseases that affect survival, including: uncontrolled malignant tumors that have metastasized and cannot be removed, blood diseases, cachexia, active

Exclusion criteria: 1. severe vomiting and difficulty in taking oral medication or absorbing in oral medication;
2. pregnant and nursing women;
3. subjects received lopinavir/ridonavir, ribavirin, monoclonal antibodies and other specific antiviral drugs in the week prior to admission;
4. subjects exist respiratory failure requiring mechanical ventilation;
5. subjects of

Exclusion criteria: The investigator considers that the subject has other conditions that make him/her unsuitable to participate in the clinical trial or other special circumstances.

Exclusion criteria: 1. In the six months before the diagnosis of 2019-nCoV acute respiratory disease, the patient was clearly diagnosed as a psychiatric disorder, including depression, bipolar disorder and so on;
2. Patients with psychotic symptoms;
3. A history of suicide attempt in the 6 months before the study, or a score of more than 3 on item 3 (suicide item) of the

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Exclusion criteria: No

Exclusion criteria: 1. Patients who are participating in other drug clinical trials;
2. Pregnant or lactating women;
3. ALT / AST> 5 ULN, neutrophils <0.5, platelets less than 50;
4. Expected survival is less than 1 week;
5. Patients diagnosed with rheumatic immune related diseases;
6. Take orally anti rejection drugs or immunomodulatory drugs for a long time;

Exclusion criteria: 1. Any situation that does not allow the program to proceed safely;
2. The patient may be transferred to a non-participating hospital within 72 hours;
3. Those who have continued to use immunosuppressive agents or organ transplants for the past 6 months;
4. Glucose-6-phosphate dehydrogenation Enzyme deficiency (faba bean

Exclusion criteria: 1) ARDS caused by other viral infections;
2) ARDS caused by other non-viral infections (such as trauma);
3) WHO grade III or IV pulmonary hypertension;
4) Patients with malignant tumors of the lungs and other systems;
5) Positive blood pregnancy test in lactating women or screening women;
6) Idiopathic IgA deficiency;
7)

Exclusion criteria: 1. Allergic constitution (persons who are allergic to more than 2 kinds of substances), people who are allergic to similar drugs in the past or those who are known to be allergic to the ingredients and auxiliary materials in the test drugs;
2. Patients who have participated in other clinical trials in the past 3 months;
3. Patients not suitable for the clinical

Exclusion criteria: (1) with severe liver or kidney failure or other serious underlying diseases;
(2) Pregnant and lactating women;
(3) Persons who are participating in other clinical trials.

Exclusion criteria: 1. Allergic to vitamin C;
2. Cardiogenic dyspnea;
3. Life expectancy<24h;
4. Pregnant and/or lactating female;
5. There is a state of tracheotomy or home oxygen therapy in the past;
6. Previously complicated with interstitial lung disease, malignant tumors diffuse alveolar hemorrhage, diabetic ketoacidosis, or an active

Exclusion criteria: Subjects who meet one of the following conditions will not be eligible for the trial:
1. pregnant or lactating women;
2. Malignant tumors, patients with other severe systemic diseases and mental illness;
3. Those who have recently participated in other clinical trials;
4. There is evidence of drug addiction within one year before joining the trial;

Exclusion criteria: 1. The patient has other immune system, blood system, heart, liver, kidney and other important organs of the disease;
2. Accompanied by chronic diseases, such as malignant tumors, chronic obstructive pulmonary disease;
3. have a history of mental illness;
4. pregnant and nursing women;
5. less than 18 years old;
6

Exclusion criteria: (1) Immunodeficiency disease, or those who have used immunosuppressive agents or glucocorticoids within the past 3 months;
(2) Preparing pregnant, pregnant and lactating women;
(3) Patients with allergies (referring to allergies to two or more medicines, foods or known ingredients in this study);
(4) Patients with mental illness, or have no

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Exclusion criteria: 1. History of tumor;
2. Vascular embolism and pulmonary hypertension;
3. Suspected or established history of alcohol, drug or drug abuse;
4. Female patients of child-bearing age who are pregnant, breast-feeding or have a recent family planning;
5. Patients who have participated in or are participating in other drug trials within one

Exclusion criteria: 1) Any situation where the solution cannot be carried out safely;
2) Allergic constitution, allergic to plasma or drugs;
3) Being too old with severe underlying diseases that affect survival, including uncontrolled clinically significant heart, lung, kidney, digestive, hematological, neuropsychiatric, immune, metabolic or malignant tumors. Severe

Exclusion criteria: 1. Patients who are participating in clinical trials of other drugs;
2. Pregnant or lactating women;
3. ALT / AST > 2-fold ULN;
4. Expected survival time < 1 week;
5. With serious liver disease;
6. Patients taking orally anti-tumor drugs or immunomodulatory drugs for a long time;
7. Patients who are participating in other clinical trials;

Exclusion criteria: (1) It is difficult to administer to patients by oral or nasal feeding;
(2) Pregnant women, as well as patients with other malignancies such as malignant tumors, mental illnesses, and patients who are not considered suitable for study.

Exclusion Criteria:

Pregnancy, lactation and those who are not pregnant but do not take effective contraceptives measures;

Patients with malignant tumor, other serious systemic diseases and

Exclusion Criteria:

Patients with severe allergies or allergies to stem cells;

Patients with serious basic diseases that affect survival, including: blood diseases, cachexia, active bleeding, severe malnutrition, etc ;

Exclusion criteria: 1. Pneumonia caused by other reasons;
2. Have contraindications that can not participate in sports activities;
3. Mental disorders, cognitive impairments etc. can not cooperate with researchers subjectively.

Exclusion criteria: 1) Any situation where the solution cannot be carried out safely;
2) Allergic constitution, allergic to one or more foods or drugs;
3) Severe basic diseases affecting survival, including uncontrolled clinically significant heart, lung, kidney, digestive, hematological, neuropsychiatric diseases, immune diseases, metabolic diseases or

Exclusion criteria: (1) Meet the diagnostic criteria for severe new-type coronavirus infection pneumonia;
(2) Severe primary diseases that affect survival, including: uncontrolled malignant tumors, hematological diseases, and HIV that have not been metastasized in multiple places;
(3) Obstructive pneumonia, pulmonary interstitial fibrosis, alveolar proteinosis

Exclusion criteria: 1. Allergy to drugs and excipients used in this study;
2. Pregnant and lactating women and women of child-bearing years did not agree to use contraception during the experiment;
3.Have malignant tumor or hypercoagulant state;
4. Suffer from mental illness and can't cooperate well;
5. Those who have participated in or are participating in

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Exclusion criteria: 1. Patients with other serious pulmonary diseases, cardiovascular and cerebrovascular diseases, diabetes mellitus, tumor, hematopoiesis, autoimmune diseases, digestive system diseases, skin diseases, psychosis, etc.;
2. Patients who cannot complete the treatment;
3. Pregnant or lactating women

Exclusion criteria: (1) Allergic to fabiravir or abidol;
(2) ALT/AST increased 5 times higher than the upper limit of normal, or with child Pugh C;
(3) Severe patients with expected survival time < 48 hours;
(4) Pregnancy;
(5) HIV positive;
(6) Considered unsuitable by researchers

Exclusion criteria: 1. There is clear evidence of bacterial infection in respiratory tract infections caused by basic diseases such as primary immunodeficiency disease, acquired immunodeficiency syndrome, congenital respiratory malformations, congenital heart disease, gastroesophageal reflux disease, and abnormal lung development;
2. Subjects with the following

Exclusion criteria: 1. The pneumonia patients with severe new coronavirus infection met one of the following conditions: respiratory distress, RR <=30 times / min; or SaO2 / SpO2 <=93% in resting state; or arterial partial pressure of oxygen (PaO2) / concentration of oxygen (FiO2) <=300MMHG (1mmhg = 0.133kpa);
2. Pneumonia patients with severe new

Exclusion criteria: 1. No malnutrition;
2. Patients who had received antiviral drugs, immunosuppressive drugs and cytotoxic drugs 2 weeks before diagnosis;
3. patients with hypertriglyceridemia (TG): Serum TG > 3.0mmol/l;
4. patients with severe liver and kidney dysfunction;
5. pregnant women; 6 tumor patients

Exclusion criteria: (1) Age < 18 or > 65;
(2) Patients with severe heart, brain, lung, kidney dysfunction, endocrine disease, hematopoiesis system disease or other serious diseases and psychosis;
(3) Pregnant and lactating women.

Exclusion criteria: 1. Serious basic diseases affecting survival, including: uncontrolled malignant tumor with multiple metastases that cannot be removed, hematological diseases, cachexia, active hemorrhage, severe malnutrition, AIDS, etc.;
2. Obstructive pneumonia, severe pulmonary interstitial fibrosis, alveolar proteinosis and allergic alveolitis caused by lung

(1) Critical patients.

(2) Patients who can not guarantee compliance of using TCM during the treatment period, or patients who are difficult to take medicine by oral or nasal route.

(3) Patients with severe primary respiratory disease or other pathogenic

Exclusion criteria: 1. AST and ALT> 1.5 x ULN at visit 1;
2. bilirubin> 1.5 x ULN at visit 1;
3. At visit 1, calculated by Cockcroftâ€“Gault formula, creatinine clearance <30 mL / min;
4. Patients with potential chronic liver disease (Child Pugh A, B or C liver injury);
5. Previous treatment with nidanib or pirfenidone;
6. One month before the screening visit (Visit 1)

Exclusion criteria: 1. Patients undergoing general gastroscopy;
2. With trachea dysfunction;
3. pregnant women, children, age more than 80 years;
4. Patients with difficulty in intubation.

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Exclusion criteria: 1.Serious basic diseases affecting survival, including: uncontrolled malignant tumor with multiple metastases that cannot be removed, hematological diseases, cachexia, active hemorrhage, severe malnutrition, AIDS, etc.;
2.Obstructive pneumonia, severe pulmonary interstitial fibrosis, alveolar proteinosis and allergic alveolitis caused by lung

Exclusion criteria: Doctors and researchers consider that the patient inappropriate to enroll in the study.

Exclusion Criteria

Cohort 1 and 2:

Subjects who meet any of the following exclusion criteria are not to be

Exclusion criteria: 1. The clinical classification of patients with severe novel coronavirus infection is to meet any of the following:
1) Respiratory failure occurs and requires mechanical ventilation;
2) Shock occurs;
3) Combined failure of other organs requires ICU monitoring and treatment;
2. Those who are allergic to blood products or plasma

Exclusion criteria: 1. Those who are allergic to Xuebijing injection and its components;
2. Pregnant or lactating women;
3. Serious basic diseases affecting survival, including: malignant tumor, hematopathy, HIV, etc;
4. Expected death within 48 hours;
5. Patients who are participating in other clinical trials;
6. The researchers think other

Exclusion criteria: 1. Cannot take medicine by mouth;
2. Those who are allergic to bismuth potassium citrate capsules or who have taken bismuth preparations within 1 month;
3. Critical patients meet one of the following conditions:
(1) Respiratory failure occurs and requires mechanical ventilation;
(2) Shock occurs;
(3) ICU monitoring and

Exclusion criteria: 1. Patients with allergic history to 4-aminoquinoline (such as chloroquine, hydroxychloroquine, piperaquine, aminophenquine, etc.) and similar drugs (such as primaquine, etc.) were identified;
2. Patients with blood system diseases;
3. Patients with hearing impairment;
4. Patients with known severe liver disease, basic disease of liver cirrhosis or

Exclusion criteria: 1. Heart rate: Heart rate <40 or> 130 beats / min;
2. New onset of myocardial ischemia or arrhythmia;
3. Blood pressure: mean arterial pressure (MAP) <65mmHg or> 110mmHg;
4. Body temperature: >=38.5 degree C;
5. Blood oxygen saturation <=93%;
6. Hemodynamic instability;
7. Severe liver and kidney dysfunction

Exclusion criteria: 1) patients in coma, unable to carry out voluntary activities and social activities;
2) patients with mental illness or adverse reactions caused by drugs;
3) patients with serious diseases such as cardiovascular, cerebrovascular, liver and kidney damage.

Exclusion criteria: Patients with severe cardiovascular disease, lung disease, sleep disorder, psychological disease, musculoskeletal system disease and other sports contraindications.

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Exclusion criteria: 1. Intubation required immidiately in case of cardiac arrest or asphyxia; 2. Pregnancy, severe chronic respiratory, uncontrolled gastric reflux disease and any nasopharyngeal anatomical obstacle; 3. Those patients has intubated in ICU.

Exclusion criteria: 1) patients in coma, unable to carry out voluntary activities and social activities;
2) patients with mental illness or adverse reactions caused by drugs;
3) patients with serious diseases such as cardiovascular, cerebrovascular, liver and kidney damage.

Exclusion criteria: 1. Patients with mild and critical novel coronavirus pneumonia;
2. Acute respiratory disease caused by non- novel coronavirus pneumonia;
3. Patients with diabetes and hypertension accompanied by severe primary immunodeficiency disease acquired immunodeficiency syndrome congenital respiratory malformation congenital

Exclusion criteria: (1) Female patients during pregnancy or lactation;
(2) Patients allergic to lipoic acid or similar drugs (B vitamins), previously intolerant to the recommended dose of lipoic acid;
(3) Patients who are taking part in clinical trials of other drugs;
(4) Patients are taking other foods or drugs with antioxidant effect, such as vitamin C, vitamin E, etc

Exclusion criteria: (1) patients with severe heart, liver, kidney and other complications or other serious primary disease or mental illness;
(2) patients whose survival time is not expected to exceed 48 hours from screening;
(3) endotracheal intubation and mechanical ventilation had been performed during screening;
(4) any influence on inclusion judged

Exclusion criteria: 1) Patients with mild, severe or critical novel coronavirus disease;
2) According to the researcher's judgment, previous or current diseases may affect patients' participation in the trial or influence the outcome of the study, including: malignant disease, autoimmune disease, severe malnutrition, liver and kidney disease, blood disease, nervous system

Exclusion criteria: 1) Patients with mild, severe or critical novel coronavirus disease;
2) According to the researcher's judgment, previous or current diseases may affect patients' participation in the trial or influence the outcome of the study, including: malignant disease, autoimmune disease, severe malnutrition, liver and kidney disease, blood disease, nervous system

Exclusion criteria: 1. Any situation where the programme cannot be carried out safely;
2. Patients who have used interferon or remedesivir;
3. No clinical manifestations and chest imaging findings;
4. Known allergy or hypersensitivity to interferon (including asthma);
5. Disabled in patients with uncontrolled autoimmune diseases;
6. Patients with severe

Exclusion criteria: 1. Patients who are allergic to corticosteroid;
2. Patients who are diagnosed with adrenal insufficiency;
3. Severe immunosuppression, one of the following: infection with human immunodeficiency virus and CD4 cell count below 350 cells per microliter, immunosuppressive therapy after solid organ transplantation, neutropenia

Exclusion criteria: (1) asthma requiring daily treatment, any other chronic respiratory disease
(2) patients with severe mental illness and severe neurosis.?
(3) the risk stratification of exercise rehabilitation was assessed as high-risk patients or those who were restricted from activities for other reasons;
(4) patients with severe renal insufficiency and liver

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Exclusion criteria: ? Doctors decided that it was not in the best interests of the subjects to participate in the study, or there was any situation that could not follow the agreement safely; ? there was a history of pulmonary embolism; ? there were patients with human immunodeficiency virus (HIV) infection; ? there were severe organ failure; such as severe liver disease

Exclusion Criteria:

With active tuberculosis, idiopathic pulmonary fibrosis, bronchial asthma, bronchiectasis, pulmonary embolism, patients with chronic respiratory failure or other severe respiratory disease;

Exclusion criteria: 1. Sever cases with shock, acute respiratory distress syndrome;
2. Multiple organ failure;
3. Pregnant or lactating women;
4. Cases with malignant disease, autoimmune disease or sever diseases of immune immune system, such as: HIV infection;
5. Major organ transplantation;
6. Other conditions that are not suitable for

Exclusion criteria: (1) Known or suspected allergy to the components of zedoary turmeric oil injection, or allergic constitution;
(2) Mild and severe patients.
(3) Combined with severe liver disease (such as Child-Pugh score >=C, TBIL >=2 times the upper limit of normal, ALT/AST >=5 times the upper limit of normal\;
(4) Combined with severe renal insufficiency

Exclusion criteria: 1. Patients with serious primary diseases such as severe mental and psychological disorders, heart and brain vessels diseases, liver and kidney diseases, endocrine diseases, hematopoietic system diseases and malignant tumors;
2. Patients are known to be allergic to trail drugs or a certain Chinese medicine of this study;
3. Women are

Exclusion criteria: (1) Combined with high blood pressure, diabetes, coronary heart disease and other underlying disease;
(2) Combined with malignant tumor or a medical history of malignant tumor;
(3) Pregnant or lactating women;
(4) Suffering from severe mental illness;
(5) Medical history of unstable angor pectoris or myocardial?infarction within 6

Exclusion criteria: 1. Patients with cardiopulmonary resuscitation at the end of the disease;
2. Pregnant women.

Exclusion criteria: 1. There is clear evidence of bacterial infection in respiratory tract infection caused by basic diseases such as primary immunodeficiency disease, congenital respiratory tract malformation, congenital heart disease, gastroesophageal reflux disease and pulmonary dysplasia;
2. Subjects with the following conditions: asthma requiring

Exclusion criteria: (1) Chronic obstructive pulmonary disease;
(2) Cardiorenal insufficiency;
(3) Pregnant women

Exclusion criteria: (1) pregnant or breast-feeding women;
(2) ALT/AST > 5 times ULN, neutrophils < 0.5x10^9/L, platelets less than 50x10^9/L;
(3) definite diagnosis of rheumatoid immunity, malignant tumor and other related diseases;
(4) patients with long-term oral immunoregulatory drugs or who had used large doses of glucocorticoids within 14 days;
(5) allergic

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Exclusion criteria: Patients with hypersensitivity to plasma products; patients with severe transfusion reactions in the past; patients with acute pulmonary edema, congestive heart failure, pulmonary embolism, malignant hypertension, polycythemia vera, extreme renal failure and other diseases.

Exclusion Criteria:

Cases of severe vomiting that make it difficult to take the drug orally
Allergic to Favipiravir or tocilizumab
Pregnant and lactating women

Exclusion criteria: Patients with difficult airways;Patients who need to remain tracheal intubation to return to the ward after surgery;After signing the informed consent, those who were conscious and awake during the perioperative period refused to continue to participate in the test.

Exclusion Criteria:

Unable to obtain informed consent;
Patients with severe liver dysfunction (Child Pugh score ≥ C, or AST > 5 times of the upper limit), severe renal dysfunction (estimated glomerular

Exclusion criteria: 1. Participation in the study is not in accordance with the rights and interests of the patient based on PIs judgement, or any other circumstances that investigators consider inappropriate for participation;
2. With bleeding or bleeding associated with severe coagulation disorders (except for disseminated intravascular coagulation unrelated to

Exclusion criteria: 1. Participation in the study is not in accordance with the rights and interests of the patient based on PIs judgement, or any other circumstances that investigators consider inappropriate for participation;
2. Low body weight patients (females<45kg, males<57kg);
3. With bleeding or bleeding associated with severe coagulation disorders (except for

Exclusion criteria: 1. The doctor believes that the patient is not suitable to participate in this trial, including those who may not cooperate, do not comply with the requirements of the procedure, or participate in this trial may put the patient in an unsafe situation.
2. Pregnant or lactation periods women;
3. Immunoglobulin allergy;
4. Immunoglobulin A deficiency;
5.

Exclusion criteria: 1. Critical new pneumonia infection that meets one of the following conditions: respiratory failure occurs and requires mechanical ventilation; shock; combined organ failure requires ICU monitoring and treatment
2. Known allergies to Ixekizumab or antiviral drugs;
3. ALT / AST> 5 times ULN, neutrophils <0.5x10^9/L, Platelets less than

Exclusion criteria: 1. Aged < 7-year-old;
2. previous arrhythmia;
3. previous chronic respiratory failure caused by other diseases, such as heart failure, thoracic deformity, structural lung disease, etc.;
4. hemodynamic instability;
5. severe immune deficiency or recent use of immunosuppressive agents;
6. allergic patients;
7. estimated

Exclusion criteria: 1. Cases with severe vomiting and difficulty in taking drugs orally or causing difficulty in taking drugs after oral administration;
2. Pregnant and lactating women;
3. The subjects participated other clinical trials within 30 days before admission;
4. Cases with no hope of survival, hospice care only, or deep coma and no response to supportive treatment

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Exclusion criteria: 1. Patients with severe primary diseases such as heart, kidney, lung, endocrine, blood, metabolism, and gastrointestinal tract, etc., may affect participation in the trial or may affect the outcome of the study by the judgment of the researcher;
2. Patients who have a family history of mental illness or have had a mental illness;
3. Patients who are allergic

Exclusion criteria: 1. With complications of multiple organ failure;
2. Patients with malignant tumor;
3. Pregnant women;
4. HIV positive.

Exclusion criteria: (1) Death is expected within 48 hours;
(2) Basic diseases such as primary immunodeficiency disease, acquired immunodeficiency syndrome, congenital respiratory malformation , congenital heart disease, gastroesophageal reflux disease, congenital pulmonary airway malformation and more with the exact evidence of

Exclusion criteria: 1. With contraindications of Ulinastatin Injection;
2. Pregnant or lactating women;
3. Patients taking other urinary trypsin inhibitors;
4. Not suitable to participate in the clinical trial by investigator's judgement.

Exclusion criteria: (1) Female patients during pregnancy or lactation;
(2) Patients with allergy to Exocarpium Citri Grandis preparation, patients with intolerance to the recommended dose of Exocarpium Citri Grandis were previously applied;
(3) Patients who are taking part in clinical trials of other drugs;
(4) Patients are taking other foods or drugs with antitussive

Exclusion Criteria:

Allergies, those who are known to be allergic to research drugs or drug excipients;
The patient weighs less than 40 kg;

Exclusion criteria: 1. people who are allergic to pirenidone capsules;
2. the researchers judged that the compliance of the subjects was not good;
3. those who cannot take drugs through gastrointestinal tract;
4. any significant clinical or laboratory abnormalities that may affect the safety assessment, such as: serum creatinine > 2mg/dl, or 176.8mmol sputum alt

Exclusion criteria: 1. Gestational female patients;
2. Patients with other malignant diseases such as malignant tumours, chronic liver or kidney diseases, and reaching the end stage;
3. Patients with known psychiatric disorders;
4. Patients withdrawing from other clinical trial within 4 weeks.

Exclusion criteria: 1. Gestational female patients;
2. Patients with allergic constitution, specifically with a history of allergies to these three intervention drugs;
3. Patients with other malignant diseases such as malignant tumours, chronic liver or kidney diseases, and reaching the end stage;
4. Patients with known psychiatric disorders;
5. Patients withdrawing from

Exclusion criteria: (1) Female patients who are pregnant or planning to become pregnant within 7 months;
(2) ALT is higher than 10 times the upper limit of normal value, or total bilirubin is highter than 2 times the upper limit of normal value, or the stage of decompensation of cirrhosis, or has had decompensation of cirrhosis;
(3) The number of peripheral blood

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Exclusion criteria: 1. Having one of the following infections (influenza virus, parainfluenza virus, adenovirus, respiratory syncytial virus, rhinovirus, human partial lung virus, SARS coronavirus and other known viral pneumonia);
2. Mycoplasma pneumoniae, Chlamydia pneumoniae and bacterial pneumonia;
3. There are non-infectious diseases, such as vasculitis

Exclusion criteria: The clinical classification of patients with severe novel coronavirus infection is to meet any of the following:
1) Respiratory failure occurs and requires mechanical ventilation;
2) Shock occurs;
3) Combined failure of other organs requires ICU monitoring and treatment;
2. Those who are allergic to blood products or plasma

Exclusion criteria: 1. Patients diagnosed with the critical type of COVID-19 [In accordance with the diagnostic criteria of the Notice on Printing and Distributing COVID-19 Diagnosis and Treatment Plan (Trial Implementation Version 6th) issued by the National Health and Medical Commission], and with unstable vital signs;
2. Patients with severe primary disease of liver

Exclusion criteria: (1) Known or suspected allergy to the components of the study drug;
(2) Acute infection of other viruses and bacteria has not been cured;
(3) The function of liver and kidney was abnormal (ALT/AST>=2 times of the upper limit of normal value, SCr>1.5 times of the upper limit of normal value);
(4) Participating in clinical trials of other

Exclusion criteria: 1. sNCP caused by other non-viral infections (such as trauma);
2. WHO grade III or IV pulmonary hypertension;
3. Malignant tumor in lung and other systems;
4. Lactating women or screening women with positive blood pregnancy test;
5. Idiopathic IgA deficiency;
6. Recent cardiovascular and cerebrovascular events (3

Exclusion Criteria:

Severe vomiting or difficulty ingesting medication
Woman who are pregnant or during lactation
Patients received lopinavir/ridonavir, ribavirin, interferon and monoclonal

Exclusion criteria: (1) Critical patients;
(2) TCM diagnosis of yang deficiency syndrome, manifested as limb warm, thin stools, chills, etc.;
(3) Patients who cannot guarantee compliance during treatment, and patients who are difficult to administer by oral or nasal route;
(4) Patients with severe primary respiratory disease or other pathogenic microbial

Exclusion criteria: (1) Patients who are unwilling to cooperate during the treatment;
(2) Patients with neocoronary pneumonia converted to critically ill during the treatment;
(3) Patients with mental illness who have severe primary diseases such as hematopoietic system, liver, cardiovascular, lung and kidney;
(4) Those who are allergic to alcohol

Exclusion criteria: 1. Other major mental disorders;
2. Weakness affects participation in the experiment;
3. Communication barriers;
4. Already participated in regular cognitive training;
5. Take psychotropic or other drugs that are known to affect mood;
6. History of major neurological deficits, including history of stroke, transient ischemic attack or

Exclusion criteria: (1) Those who are allergic to pirfenidone;
(2) The investigator judges that the subject's compliance is poor.
(3) Patients with severe liver disease;
(4) pregnant and lactating patients;
(5) Those who have severe kidney disease or need dialysis treatment;
(6) Those who need to take fluvoxamine at the same time

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Exclusion criteria: (1) Patients allergic to chloroquine and hydroxychloroquine;
(2) Women during pregnancy;
(3) Patients with heart, lung, kidney, brain, blood and other important organs with severe diseases accompanied by insufficiency;
(4) Patients with retinal disease, hearing loss or hearing loss;
(5) Patients with severe neurological and

Exclusion Criteria:

Other types of viral pneumonia, or bacterial pneumonia.
The clinical classification is mild, moderate or critical;
Patients with malignant blood or solid tumor

Exclusion criteria: 1. severe vomiting and difficulty in taking oral medication or in taking oral medication;
2. pregnant and nursing women;
3. subjects received lopinavir/ridonavir, ribavirin, monoclonal antibodies and other specific antiviral drugs in the week prior to admission;
4. cases of respiratory failure requiring mechanical ventilation;
5. cases of shock;

Exclusion Criteria:

Have other significant diseases other than COVID-19, that is, a disease that, according to the investigator's judgment, can cause the subject to be at risk due to participation in the study, or affect the study results and the ability of

Exclusion Criteria:

Patients participating in clinical trials of other drugs;
pregnant or lactating women;
ALT / AST> 5 times ULN, or neutrophils <0.5 * 109 / L, or platelets less than

Exclusion Criteria:

patients with mild COVID-19 pneumonia
liver dysfunction
concomitant with other active infection

Exclusion criteria: (1) Basic diseases: Patients with severe diabetes, cardiopulmonary insufficiency, chronic kidney disease, chronic liver failure, cirrhosis, etc. before onset; tumor patients; patients with azotemia;
(2) Not willing to participate in the experiment;

Exclusion criteria: 1. Combining patients with severe primary respiratory diseases, or with other pathogenic microbial pneumonia that needs to be identified in relation to COVID-19;
2. Combining systems such as heart, brain, lung, liver and kidney syllading with severe primary diseases, as well as mental illness;
3. Those who are unconscious and unable to

Exclusion Criteria:

Receive any clinical trial drug treatment for COVID-2019 within 30 days before the screening assessment;
Severe liver disease (e.g., Child Pugh score >=C or AST> 5 times of the

Exclusion criteria: 1) Previous history of chronic bronchitis, emphysema, interstitial lung disease or pulmonary heart disease;
2) Combining with other serious diseases: such as those who have suffered myocardial infarction and uncontrollable diabetes within 6 months, and are considered unsuitable to participate in the trial;
3) People with active peptic ulcer;

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Exclusion criteria: 1. In pregnancy, breast-feeding female patients;
2. to determine the history of chloroquine phosphate, rabeprazole drug allergy patients;
Patients suffering from diseases of the blood system;
3. patients suffering from chronic liver and kidney diseases and reaching the terminal stage;
4. Serious heart, lung dysfunction and mental illness;

Exclusion Criteria:

Those allergic to fapilavir;
Pregnant or lactating women;
Unstable liver, kidney, and heart diseases;

Exclusion Criteria:

Combined with severe heart, lung (diagnosed with interstitial lung disease, bronchial asthma, chronic obstructive pulmonary disease, etc.), liver and kidney disease or with endocrine, rheumatic, neurologic, malignant and other

Exclusion criteria: (1) patients without definite diagnosis of novel coronavirus pneumonia;
(2) combined with a end-stage disease with a life expectancy of less than 1 year;
(3) intolerance to cardiopulmonary monitoring recorder;
(4) regular follow-up is not possible.

Exclusion criteria: 1. Patients with primary and secondary cardiovascular, cerebrovascular, renal, endocrine, neurological and blood system diseases, tumors and tuberculosis;
2. Patients who have participated in or are participating in other clinical trials within one month;
3. Patients with mental disorders who can not express their feelings;
4. There are other

Exclusion Criteria:

Family history of seizure, epilepsy, brain or mental disease
Subject allergic to any component of the investigational vaccine, or a more severe allergic reaction and history of allergies in the past

Exclusion criteria: 1) left heart failure pulmonary edema;
2) pregnancy and lactation or perinatal period;
3) liver failure MELD score >40;
4) a history of deep vein thrombosis or pulmonary embolism in the past 3 months;
5) cases that the researcher judged to be unsuitable for the group

1. Confirmed cases, suspected cases or asymptomatic cases with COVID-19 (refer to Information System of China Disease Prevention and Control);
2. COVID-19 antibody test positive;
3. Fever (armpit temperature > 37.0 degree C), dry cough, fatigue, nasal obstruction, runny nose, sore throat, myalgia, diarrhea, shortness of breath

Exclusion criteria: 1. Dyspnea scale mMRC 0~1 or 4;
2. Resting heart rate>100bpm;
3. Adjusting dosage of drugs which affect cardiopulmonary function or heart rate (such as: trimetazidine, salbutamol, beta-blocker);
4. Combined uncontrolled chronic disease, such as uncontrolled hypertension (resting BP=160100mmHg), uncontrolled diabetes

Exclusion criteria: 1. Patients who are difficult to be administered orally or by nasal feeding, those who are allergic to the test drug and those who are intolerant to the drug;
2. The compliance of medication cannot be guaranteed during the treatment;
3. Patients with severe primary respiratory diseases, or pneumonia caused by other pathogen;
4.

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Exclusion Criteria:

Travel history / residence history of Wuhan city and surrounding areas, or other communities with case reports within 14 days;
History of contact with a SARS-CoV-2 infection (positive in nucleic acid test)

Exclusion criteria: (1) People with severe primary diseases such as heart, brain, liver, kidney and hematopoietic system;
(2) People with pulmonary malignancy;
(3) Combined with mental, intellectual or language disorders;
(4) Pregnant women, lactating women and those who have pregnancy requirement in the last 6 months;
(5) People who cannot

Exclusion criteria: (1) Patients diagnosed with COVID-19;

(2) positive nucleic acid test;

(3) positive IgM of SARS-CoV-2;

(4) positive IgG of SARS-CoV-2;

(5) pregnant or preparing pregnant women, lactating women, men

Exclusion criteria: 1. pregnancy or lactation;
2. having a history of major diseases such as disorders of liver, kidney, endocrine or hematological systems;
3. coagulation disorder;
4. explicit history of tumors;
5. having a history of malnutrition and cachexia;
6. contraindicated by HBOT, such as pneumothorax, poor pressure adjustment of the

Exclusion criteria: 1) Patients with specific contraindications of ulinastatin;
2) Female subjects had a positive urine pregnancy test or were lactating during the screening period;
3) Patients who are using Civerex and Gabexate;
4) Factors that may increase the mortality rate, which patients with severe other basic diseases such as tumor, severe malnutrition

Exclusion Criteria:

Surgery in the lung that affects lung function;
Rely on mechanical ventilation to maintain lung function;
Chronic lung diseases affecting lung function;

Exclusion criteria: 1. History of pulmonary surgery which affected lung function;
2. Relying on mechanical ventilation to maintain lung function;
3. Complications of chronic lung disease, such as COPD;
4. Complications affecting heart function;
5. Patients with severe primary diseases may affect participation in the trial or may affect the outcome of the

Exclusion criteria: 1. Because of other reasons, such as shortness of breath, hyperhidrosis, dry cough, chest distress, palpitation, etc;
2. Patients with difficulty in taking drugs orally due to basic diseases;
3. There are serious basic diseases that affect survival, including uncontrolled clinically significant heart, lung, kidney, digestion, blood disease, neuropsychiatric

Exclusion criteria: 1. Patients with difficulty in taking oral drugs due to basic diseases;
2. Accompanied by severe basic diseases affecting survival, including uncontrolled clinically significant heart, lung, kidney, digestion, blood diseases, neuropsychiatric diseases, immune diseases, metabolic diseases, malignant tumors, and severe malnutrition;
3. Allergic

Exclusion criteria: 1. Other viral pneumonia;
2. Patients who received tumor immunotherapy (such as PD-1 / L1, CTLA4, etc.) and inflammatory factor regulators such as ulinastatin in the past 1 month;
3. Patients who have used macrolides and other antibacterial drugs in the past week;
4. Patients who have undergone organ transplantation or surgery planning

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Exclusion Criteria:

Women who have recently been pregnant or breast-feeding.
Having a history of active gastrointestinal bleeding in the past 3 months.
Blood routine examination showed that the platelet count was < 30×10^9/l

Exclusion criteria: (1) Patients with fasting water due to gastrointestinal bleeding, intestinal obstruction, etc;
(2) Patients with pre-existing severe brain, heart, liver, kidney and hematopoietic system diseases;
(3) Patients who are known to be allergic to the ingredients of the intervention drug;
(4) Patients who are unable to cooperate with follow-up

Exclusion criteria: 1. Allergic to Aliskiren or Nifedipine;
2. Pregnant or breastfeeding;
3. History of Aliskiren related angioedema;
4. Severe kidney disease or other organs failure need ICU care;
5. Cardiogenic shock or KOCK utricle;
6. Participation in anther clinical studies within 3 months of enrollment

Exclusion criteria: (1) patients with severe diseases such as cardiovascular, cerebrovascular, hematopoietic, digestive system or mental illness;
(2) pregnant and lactating women;
(3) respiratory frequency> 30 times /min, showing respiratory failure;
(4) complicated with other organ failure requires treatment by respiratory intensive care unit (ICU);
(5)

Exclusion criteria: 1. Patients with difficulty in taking oral drugs due to basic diseases;

2. Accompanied by severe basic diseases affecting survival, including uncontrolled clinically significant heart, lung, kidney, digestion, blood diseases, neuropsychiatric diseases, immune diseases, metabolic diseases, malignant tumors, and severe malnutrition;

3

Exclusion criteria: (1) Pregnant women;
(2) resting heart rate <40 or >130 beats/min;
(3) unstable angina or arrhythmia;
(4) mean arterial pressure <65mmHg or >110mmHg;
(5) peak body temperature >=38 degrees C;
(6) resting blood oxygen saturation <=93%;
(7) mechanical ventilation;
(8) respiratory rate > 40 times/min or < 5

Exclusion criteria: 1. Confirmed cases, suspected cases or asymptomatic cases with COVID-19 (refer to Information System of China Disease Prevention and Control);
2. Positive in serum antibodies (IgG and IgM) screening of COVID-19;
3. Has a history of SARS virus infection (self-reported, site information);
4. Fever (armpit temperature > 37.0 degree

Exclusion criteria: 1. Patients allergic to chloroquine and hydroxychloroquine;
2. Pregnant women;
3. Patients with severe diseases of heart, lung, kidney, brain, blood and other important organs and dysfunction;
4. Patients with retinopathy, hearing loss or hearing loss;
5. Patients with severe neurological and psychiatric diseases;
6. Subjects who, in

Exclusion criteria: 1. Known or suspected allergy to the components of BFHX.
2. Acute infections, along with other viruses or bacteria.
3. Abnormal liver and kidney function tests (ALT, AST, SCr >= 1.5 times the upper limit of normal).
4. Participating in other drug clinical trials.
5. Pregnant and lactating women; or those who cannot take effective

Exclusion criteria: 1. Accompanied by severe basic diseases affecting survival, including uncontrolled clinically significant heart, kidney, digestive, blood diseases, neuropsychiatric diseases, immune diseases, metabolic diseases, malignant tumors, severe malnutrition, etc.;
2. Unstable angina pectoris or myocardial infarction in the last 1 month;
3. Allergic

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Exclusion criteria: (1) Critical type of COVID-19 according to COVID-19 diagnosis and treatment guidelines in China (7th edition);
(2) Gastrointestinal perforation;
(3) Gastrointestinal obstruction;
(4) Recent history of intestinal fistula;
(5) Disturbance of consciousness;
(6) Gastrointestinal hemorrhage;
(7) Malignant tumors;
(8)

Exclusion criteria: (1) known or suspected allergies to the components of azivudine tablets;
(2) according to the latest version of the National Health and Medical Commission Diagnostic criteria of COVID-19, ?patients with severe novel coronavirus pneumonia was confirmed;
(3) severe liver disease (TBIL >=2 times normal upper limit; ALT,AST>=5 times normal

Exclusion Criteria:

Travel / residence history of Wuhan city and surrounding areas or other communities with case reports within 14 days prior to the enrolment;

Exclusion Criteria:

Contact with SARS-CoV-2 infected persons (positive for nucleic acid

inflammatory bowel disease;
have other sources of infection;
death is anticipate within 72 hours;

Exclusion criteria: 1. Pregnant women within 3 months of pregnancy;
2. Patients with liver and kidney disease and those with liver and kidney injury
3. Patients with blood diseases (such as leukopenia, thrombocytopenia, etc.).
4. Electrocardiogram prompting patients with severely extended QT

Exclusion criteria: 1. Patients with fatigue, anorexia, abdominal distension, loose stools due to other reasons or diseases of the digestive system;
2. Patients with difficulties in taking oral drugs due to underlying diseases and other reasons;
3. It is accompanied by serious basic diseases that affect survival, including uncontrolled clinically meaningful heart, lung, kidney

Exclusion criteria: 1. Past history of psychiatric disease or be receiving psychiatric treatment;
2. Past personality disorder, intellectual disability, brain injury or brain disease;
3. With severe cardiovascular, cerebrovascular, neuromuscular and other system diseases or other diseases that injure physical activity;
4. Past history of tumor;
5

Exclusion criteria: 1. Severe pneumonia requires mechanical ventilationcritically severe cases;
2. Estimated Time of Death is less than 48 hours;
3. There is clear evidence of bacterial infection in respiratory tract infections caused by basic diseases such as primary immunodeficiency disease, acquired immunodeficiency syndrome, congenital

Exclusion Criteria:

Phase Ia:

Contraindications for vaccination

Exclusion criteria: 1. Patients with digestive disease previously;
2. Patients with mental disorders who cannot communicate normally;
3. Patients without complete medical information.

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Exclusion criteria: 1. Patients with bronchiectasis, tuberculosis, asthma, chronic obstructive pulmonary disease, lung cancer and other primary lung diseases;
2. Progressive exacerbation of dyspnea symptoms in the past 6 months;
3. In the acute exacerbation stage of pulmonary fibrosis;
4. The random blood glucose level was more than 11.1mmol/l;
5.

Exclusion criteria: 1. The physician determines that participation in the study is not in the patient's interest, or in any other case that does not allow the program to proceed safely;
2. Severe liver disease, underlying cirrhosis or a five-fold increase in Alanine transaminase Aspartate transaminase; 3. Severe impairment of Renal Function (estimated glomerular filtration rate

Exclusion criteria: (1) patients with severe diseases such as cardiovascular, cerebrovascular, hematopoietic, digestive system or mental illness;
(2) pregnant and lactating women;
(3) respiratory frequency> 30 times /min, showing respiratory failure;
(4) complicated with other organ failure requires treatment by respiratory intensive care unit (ICU);
(5)

Exclusion criteria: 1. Those who do not meet the diagnostic criteria for new coronary pneumonia;
2. Women who are pregnant or breastfeeding, who are allergic to this medicine;
3. Combining patients with severe primary diseases such as cardiovascular, liver, kidney and hematopoietic system, mental illness;
4. Contains factors that affect the observation of

Exclusion Criteria:

Excluded for novel coronavirus (SARS-CoV-2) infection

Having one of the following evidence on SARS-CoV-2 infection:

Exclusion criteria: 1. Excluded who diagnosed as pneumonia patients with novel coronavirus infection of severe type and critical type. Diagnostic criteria reference the official guideline "Pneumonia Diagnosis and Treatment Scheme for Novel Coronavirus Infection (Trial Version 6)";
2. Patients with allergic to the ingredients of bromhexine hydrochloride tablets and/or

Exclusion Criteria:

The vital signs, physical examination and laboratory test indicators of the population specified in the plan are abnormal and have clinical significance as determined by the clinician;

1. Previously diagnosed with depression and currently taking medication
2. Prior cognitive dysfunction
3. Experienced another major stressful event (e.g. divorce, bereavement) in the past year

Exclusion criteria: Subjects who meet any of the following exclusion criteria may not be enrolled:
1. Subjects who do not meet the health standards as assessed through comprehensive physical examination, mainly including:
(1) With abnormal and clinically significant vital signs (pulse rate under conscious state < 50 beats/min or > 100 beats/min, systolic blood pressure <

Exclusion Criteria:

The vital signs and physical examination results of the population specified in the plan have clinical significance as determined by the clinician;

A history of severe allergies to any component of the test vaccine, including

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Exclusion Criteria:

Phase Ib:

Contraindications for vaccination
Exclusion Criteria:

Any clinically significant chronic or acute medical condition that makes the volunteer unsuitable for participation;
A history of significant hypersensitivity, intolerance, or allergy to any drug
Exclusion Criteria:

Any clinically significant chronic or acute medical condition that makes the volunteer unsuitable for participation;
A history of significant hypersensitivity, intolerance, or allergy to any drug
Exclusion Criteria:

Having a history of severe allergy, such as severe allergic reactions, urticaria, angioedema;
Having one of the following evidence on SARS-CoV-2 infection(previous
Exclusion criteria: 1. Those who have had lung surgery that affects lung function;
2. Those who rely on mechanical ventilation to maintain lung function;
3. Combined with chronic lung and kidney disease affecting lung and kidney function;
4. Combined with diseases that affect heart function;
5. Accompanied by serious underlying diseases that affect
Exclusion criteria: 1) Health status: medical history of heart, liver, kidney, digestive tract, nervous system, respiratory system, blood and lymphatic system, immune system, mental and metabolic abnormalities, bone and other diseases with clinical significance;
2) Allergy to biological agents or any drug components:Those who had a history of allergy and were judged
Exclusion criteria :

The results of physical examination and laboratory examination before screening are abnormal and clinically significant as judged by clinicians, or systolic blood pressure >160mmHg and/or diastolic blood pressure
Exclusion Criteria:

For adult group (age ≥18 and ≤55)

Have had any acute illness, as determined by the investigator, with or without
Exclusion Criteria:

Health status: clinically significant histories of heart, liver, kidney, digestive tract, nervous system, respiratory system, blood and lymphatic system, immune system, mental, metabolic, and bone abnormalities
Exclusion Criteria:

Exclusion criteria of prime dose:

Subjects with a medical or family history of convulsions, epilepsy

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Exclusion criteria: 1. Patients with severe mental illness or suicidal tendency;
2. Patients with serious heart, brain, lung, kidney, liver and other medical diseases or tumors;
3. Patients who can not cooperate with the investigation and intervention;
4. Pregnant or lactating women.
Those who meet any of the above items shall be excluded

Exclusion criteria: 1) Subaxillary body temperature >37.0 degree C;
2) A history of close contact with patients with COVID-19 within 21 days before vaccination;
3) Those with a history of severe acute respiratory syndrome (SARS) or Middle East respiratory syndrome (MERS);
4) Women with positive blood pregnancy test, or women in pregnancy or

Exclusion criteria: 1. Patients with any following positive 2019-nCoV screening results should be excluded:
(1) Serum antibodies (IgG and IgM) of the new crown were positive;
(2) Lung CT showed new coronal imaging features;
(3) The results of RT-PCR were positive.
2. HCV antibody, HBsAg, TP antibody, HIV antibody or tuberculosis positive (lung CT

Exclusion Criteria:

Travel history / residence history of communities with case reports within 14 days;
History of contact with a SARS-CoV-2 infection (positive in nucleic acid test)

Exclusion criteria: 1. The patients who are fasting water due to gastrointestinal bleeding and intestinal obstruction;
2. Patients with severe diseases of brain, heart, liver, kidney and hemopoietic system could not cooperate with each other, the ALT of liver function was more than 1.5 times of normal upper limit;
3. Patients who are known to be allergic to

Exclusion criteria: 1. The patients who are fasting water due to gastrointestinal bleeding and intestinal obstruction;
2. Patients with severe diseases of brain, heart, liver, kidney and hemopoietic system could not cooperate with each other, the ALT of liver function was more than 1.5 times of normal upper limit;
3. Patients who are known to be allergic to

Exclusion Criteria:

Family history of seizure, epilepsy, brain or mental disease
Subject allergic to any component of the investigational vaccine, or a more severe allergic reaction and history of allergies in the past

Exclusion Criteria:

positive IgM/IgG against SARS-CoV-2 prior to randomization.
Severeor critical illness
Uncontrolled hypertension, cardiovascular/cerebrovascular diseases,lung

Exclusion criteria: 1. Patients with fasting water due to gastrointestinal bleeding, intestinal obstruction, etc.;
2. Those who cannot cooperate with the original severe brain, heart, liver, kidney and hematopoietic system diseases; those with malignant tumors and mental and neurological disorders who cannot cooperate;
3. People with allergies or allergic to the

1. Within 14 days before vaccination, subjects have been abroad and to villages/communities experienced COVID-19 epidemics, and in contact with COVID-19 cases or suspected cases. Subjects are under isolation observation, or living in the villages/communities with COVID-19 cases or suspected cases;

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Exclusion Criteria:

1) With one of the following respiratory diseases:

Subjects with asthma history, or cannot rule out asthma based on the

Exclusion criteria: Subjects meeting any of the following exclusion criteria were not included in the study
1. The subjects who did not meet the health standards in the comprehensive physical examination mainly included:
(1) Patients with abnormal vital signs and clinical significance (awake pulse < 50 beats / min or > 100 beats / min, systolic blood pressure >= 140

Exclusion criteria: 1. Patients with mental history;
2. Patients who do not use the Internet.

Exclusion criteria: 1. Within 14 days before vaccination, subjects have been abroad and to villages/communities experienced COVID-19 epidemics, and in contact with COVID-19 cases or suspected cases. Subjects are under isolation observation, or living in the villages/communities with COVID-19 cases or suspected cases;
2. Confirmed cases, suspected cases or

Exclusion Criteria:

Travel/residence history of communities with reported cases within 14 days before trial;
History of exposure to Novel Coronavirus infection (nucleic acid positive)

Exclusion criteria: 1. Subjects with axillary body temperature above 37.0 degrees C;
2. Patients with history of severe acute respiratory syndrome (SARS) or Middle East respiratory syndrome (mers);
3. Female subjects with positive urine pregnancy test, or pregnant or lactating women;
4. Subjects with acute sinusitis or acute rhinitis within 3 days before

Exclusion Criteria:

A subject who was diagnosed with severe or critical COVID-19 (as per the Diagnosis and Treatment Protocol for Novel Coronavirus Pneumonia (Trial, 8th edition), China, or Clinical Management of COVID-19, WHO);

Exclusion Criteria:

Other medical or psychiatric condition including recent (within the past year) or active suicidal ideation/behavior or laboratory abnormality that may increase the risk of study participation or, in the investigator's judgment

Exclusion Criteria:

Have a history of close contact with a confirmed case of SARS-CoV-2, an asymptomatic infection in the previous 14 days, or a travel history/residential history in a community where a case has been reported

Exclusion Criteria:

First dose exclusion criteria:

Positive serum immunoglobulin M (IgM) and IgG to the SARS-CoV-2

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Subject can be excluded by any of the following:
1.COVID-19 antibody test is positive;
2.Those with confirmed infection of SARS or MERS virus previously.
3.Those with an acute infection within 2 weeks before vaccination.
4.Those with a history of needle sickness.

Exclusion criteria: 1. Fever is suspected or diagnosed within 72 hours before enrollment (axillary temperature >=37.3 degree C/oral temperature >=37.5 degree C), or underarm body temperature >=37.3 degree C/oral temperature >=37.5 degree C on the day of enrollment;
2. Diastolic blood pressure >=100mmHg and/or systolic blood pressure >=150mmHg;
3. Those with

Exclusion Criteria:

Patients with critical COVID-19 presenting with shock, acute respiratory distress syndrome, multiple organ failure

Patients with mild COVID-19

Exclusion criteria: 1. Pneumonia caused by other viruses, except for 2019-nCov;
2. Clear bacterial infection;
3. Subjects who have used antiviral drugs and may need another antiviral treatment during the study;
4. There are serious non-infectious pulmonary diseases, including tuberculosis, pulmonary edema and pulmonary embolism;
5. Severe

Exclusion Criteria:

Any clinically significant chronic or acute medical condition that makes the volunteer unsuitable for participation.
A history of significant hypersensitivity, intolerance, or allergy to any drug

Exclusion criteria: 1. Patients with history of allergy, convulsion, epilepsy, encephalopathy and psychosis or family history;
2. Those who are allergic to any component of the vaccine, and those who have had any history of severe allergy to the vaccine (such as acute allergic reaction, urticaria, skin eczema, dyspnea, angioneurotic edema or abdominal pain);

Exclusion criteria: 1. Subjects with history of allergy, convulsion, epilepsy, encephalopathy and psychosis or family history;
2. Patients who are allergic to any component of the vaccine, and have a history of severe allergy to the vaccine (such as acute allergic reaction, urticaria, skin eczema, dyspnea, angioneurotic edema or abdominal pain);
3. Patients with

Exclusion criteria: 1. Patients with history of allergy, convulsion, epilepsy, encephalopathy and psychosis or family history;
2. Those who are allergic to any component of the vaccine, and those who have had any history of severe allergy to the vaccine (such as acute allergic reaction, urticaria, skin eczema, dyspnea, angioneurotic edema or abdominal pain);

Exclusion criteria: 1. Those who do not meet the health standards in the comprehensive physical examination mainly include:
(1) Patients with abnormal vital signs and clinical significance (awake pulse < 50 beats / min or > 100 beats / min, systolic blood pressure >= 140 mmHg or diastolic blood pressure >= 90 mmHg);
(2) Body mass index (BMI) < 18 kg / m2 or > 30

1. There is clear evidence of bacterial infection in respiratory tract infections caused by basic diseases such as primary immunodeficiency disease, acquired immunodeficiency syndrome, congenital respiratory malformations, congenital heart disease, gastroesophageal reflux disease, and abnormal lung development;

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1. There is clear evidence of bacterial infection in respiratory tract infections caused by basic diseases such as primary immunodeficiency disease, acquired immunodeficiency syndrome, congenital respiratory malformations, congenital heart disease, gastroesophageal reflux disease, and abnormal lung development;

1. Patients with new coronavirus pneumonia confirmed by pathogenic testing and clinical manifestations or signs;
2. In the opinion of the investigatorpatients with serious diseases such as malignant disease, autoimmune disease, liver and kidney disease, blood disease, neurological disease, and endocrine disease may affect patients'

Exclusion criteria: 1.Patients who have been diagnosed with Covid-19 cases and suspected cases, have a history of contact with confirmed cases and suspected cases of new coronavirus in 14 days before signing the informed consent form, or have SARS-COV-2 infection or carriers;
2.Having diseases history of cardiovascular system, respiratory system, digestive

Exclusion Criteria:

Positive serum immunoglobulin M (IgM) and immunoglobulin G (IgG) to the SARS-CoV-2.
SARS-CoV-2 nucleic acid testing positive

1) Patients who have undergone operations that affect lung function, such as lung transplantation, lung resection, lung volume reduction, etc;
2) Combined with chronic lung diseases that affect lung function, such as severe chronic obstructive pulmonary disease;
3) Patients complicated with diseases affecting cardiac function, such as

Exclusion criteria: 1. Confirmed as a confirmed case of new coronavirus pneumonia after pathogenic testing and clinical manifestations or signs;
2. Allergies, such as those with a history of allergies to two or more drugs or foods, or those who are known to be allergic to the ingredients of this drug;
3. Women who are pregnant or breastfeeding;
4. The

Exclusion criteria: 1. Patients who have difficulty in oral medication due to underlying diseases and other reasons;
2. Patients with severe primary diseases and dysfunctions such as heart, brain, breathing, digestion, endocrinology, hematopoiesis, liver and kidney, ALT and AST are 2 times higher than the upper limit of normal, or Cr>130μmol/L ;
3. People with

Exclusion Criteria:

Recurring COVID-19 patients
Subjects with any unstable conditions, a history of significant hypersensitivity, or known allergy to components of the investigational agent

1. Subjects with history of convulsion, epilepsy, Guillain Barre syndrome, encephalopathy or mental illness or family history;
2. Fever (armpit temperature > 37.0 degree C) before vaccination;
3. Subjects with history of severe allergic reactions (such as anaphylactic shock, acute anaphylaxis, urticaria, skin eczema, dyspnea, angioneurotic

Exclusion Criteria:

Travel history / residence history of communities with case reports within 14 days;
History of contact with a SARS-CoV-2 infection (positive in nucleic acid test)

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Exclusion Criteria:

Have a history of close contact with a confirmed case of SARS-CoV-2, an asymptomatic infection in the previous 14 days, or a travel history/residential history in a community where a case has been reported

1. Patients who cannot guarantee medication compliance during treatment and who are difficult to administer medication through oral nasal feeding;
2. Patients with severe primary respiratory disease, or other pathogenic microbiological pneumonia that needs to be differentiated from COVID-19 or Novel Coronavirus asymptomatic infection;

Exclusion Criteria:

have a medical history or family history of convulsion, epilepsy, encephalopathy and psychosis.

be allergic to any component of the research vaccines, or used to have a

Subjects who meet any of the following criteria will be excluded:

1. Confirmed or asymptomatic COVID-19 cases;
2. Positive SARS-CoV-2 nucleic acid test;
3. Positive SARS-CoV-2 IgG or IgM test (reference to nucleic acid test and chest CT if necessary);

Subjects who meet any of the following criteria will be excluded:

1. Confirmed or asymptomatic COVID-19 cases;
2. Positive SARS-CoV-2 nucleic acid test;
3. Positive SARS-CoV-2 IgG or IgM test (reference to nucleic acid test and chest CT if necessary);

1. Those who are allergic to any component of the vaccine, and those who have history of severe allergic reactions to the vaccine, such as acute allergic reactions, urticaria, skin eczema, dyspnea, angioedema or abdominal pain;
2. Those with fever or those who suffer from acute diseases or severe chronic diseases in the acute stage of onset;

Exclusion Criteria:

Serious cardiovascular diseases, such as arrhythmia, conduction block, myocardial infarction, severe hypertension and uncontrollable medications (on-site measurement: systolic blood pressure ≥160mmHg, diastolic blood

Exclusion Criteria:

First Dose Exclusion Criteria：

Confirmed cases of SARS-CoV-2 infection, suspected cases, asymptomatic

Exclusion Criteria:

Confirmed cases, suspected cases or asymptomatic infections of SAR-CoV-2 infection (check "China Disease Prevention and Control Information System")

1. Have been diagnosed with a COVID-19 case or suspected case, had contact with a confirmed or suspected case of Novel Coronavirus within one month before signing the informed consent, or have traveled to areas with high epidemic incidence or abroad; Or novel coronavirus infection or carrier: serum anti-SARS-CoV-2 specific antibody positive, or nasopharyngeal swab

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Exclusion Criteria:

Travel history / residence history of communities with case reports within 14 days;
History of contact with a SARS-CoV-2 infection (positive in nucleic acid test).

1. Having a history of COVID-19 or a positive nucleic acid test for COVID-19;
2. Having a history or a family history of allergy, convulsion, epilepsy, brain disease, or mental illness;
3. Being allergic to any component of vaccines, or having a history of severe allergic reaction to any vaccine;

1. Having a history of COVID-19 or a positive nucleic acid test for COVID-19;
2. Having a history or a family history of allergy, convulsion, epilepsy, brain disease, or mental illness;
3. Being allergic to any component of vaccines, or having a history of severe allergic reaction to any vaccine;

1) Patients who have undergone operations that affect lung function, such as lung transplantation, lung resection, lung volume reduction, etc;
2) Dependent on mechanical ventilation to maintain lung function, such as ventilator;
3) Combined with chronic lung diseases that affect lung function, such as

Exclusion Criteria:

the patients who are diagnosed as critically ill cases or participating in other clinical trials
women who are pregnant or lactating

Exclusion Criteria:

Medical or family history of convulsions, epilepsy, encephalopathy, and psychosis disorders;
History of allergies to any ingredient of Ad5-nCoV, history of serious allergic

Exclusion Criteria:

have the medical history or family history of convulsion, epilepsy, encephalopathy and psychosis.
be allergic to any component of the research vaccines, or used to have a

1) Axillary temperature > 37.0 degree C;
2) Patients with Corona Virus Disease 2019 (COVID-19), severe acute respiratory syndrome (SARS) or Middle East respiratory syndrome (MERS).
3) Patients with history of COVID-19 vaccines or other candidate vaccines (except sequential subgroup);

1. The new coronavirus antibody test is positive, and the new coronavirus nucleic acid test is positive;
2. Have a history of SARS/MERS virus infection (self-report, on-site inquiry);
3. Have been vaccinated against other novel coronavirus vaccines;
4. Those who have a history of allergy to phenol and Mamemin;

1. The nucleic acid test turns negative twice in a row (24 hours apart);
2. Those who meet the discharge criteria and have a positive nucleic acid after discharge;
3. Severely and critically ill patients (8th edition standard);
4. Abnormal liver and kidney function (more than 2 times the normal value);

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Exclusion Criteria:

Individuals with fever >37.3°C (axillary), or any acute illness at baseline (Day 1) or within 3 days prior to randomization. Participants meeting this criterion may be rescheduled within the relevant window. Febrile participants with

Exclusion Criteria:

Those who have had surgery within 8 weeks prior to screening or who have surgery planned during the trial, and the investigator believes that such surgery may bring potential risks to the subjects

Exclusion Criteria:

History of SARS-CoV-2 infection;
Have received any COVID-19 vaccine;
Participants with abnormal fasting blood glucose or diabetes;

Exclusion Criteria:

The results of physical examination and laboratory examination before screening were abnormal and clinically significant;
Any component of the test vaccine, including aluminum preparation, has a

Exclusion Criteria:

Confirmed or asymptomatic COVID-19 cases or SARS-CoV-2 infection(had positive in SARS-CoV-2 nucleic acid test or serological test).
Had a history of traveling or residence in domestic area of high and moderate

Exclusion Criteria:

Confirmed or asymptomatic COVID-19 cases or SARS-CoV-2 infection(had positive in SARS-CoV-2 nucleic acid test or serological test).
Had a history of traveling or residence in domestic area of high pandemic

Exclusion Criteria:

Travel history / residence history of city and surrounding areas, or other communities with case reports within 14 days;
History of contact with a SARS-CoV-2 infection (positive in nucleic acid test)

Exclusion Criteria:

Within 14 days before vaccination, subjects have been abroad and to villages/communities experienced COVID-19 epidemics, and in contact with COVID-19 cases or suspected cases. Subjects are under isolation

Exclusion Criteria:

Within 14 days before vaccination, subjects have been abroad and to villages/communities experienced COVID-19 epidemics, and in contact with COVID-19 cases or suspected cases. Subjects are under isolation

Exclusion Criteria:

Travel history / residence history of communities with case reports within 14 days prior to the study;
History of contact with a SARS-CoV-2 infection (positive in nucleic acid test)

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Exclusion Criteria:

History of SARS-CoV-2 infection.
The COVID-19 antibody (IgG and IgM) screening was positive.
History of asthma, history of allergy to vaccines or vaccine components, or
Exclusion criteria for the first dose:
1. People who have been to a foreign country or an infected village/community within 14 days prior to inoculation and have been in contact with NOVEL CORONAVIRUS infection or suspected case, who are in isolation, or who have been infected or suspected case in the same
(1) Patients whose COVID-19 treatment regimen did not meet the discharge criteria;
(2) patients with severe primary heart, liver, kidney, blood or other diseases affecting their survival;
(3) Patients with mental illness, limited cognitive ability and legal disabilities;
(1) Aged < 18 years;
(2) Patients with severe primary heart, lung, liver, kidney, blood or diseases affecting their survival;
(3) Creatinine and ALT / AST exceeded the upper limit of normal value by 1.5 times;
(1) Nucleic acid test turned negative twice continuously (24 hours interval);
(2) those who meet discharge criteria and have positive nucleic acid test after discharge;
(3) those who are in critical condition and are expected to die within 24 hours;
(1) Close contacts who can not guarantee compliance of using TCM during the treatment period, or close contacts who are difficult to take medicine by oral or nasal route;
(2) Close contacts with severe primary respiratory disease;
(3) Pregnant or parturient women. Patient who have a pregnancy plan or a
Exclusion Criteria for Dose 1:
Subjects who meet any of the following exclusion criteria should be excluded from the study:
1. Individuals whose results from a complete physical examination do not meet the health standards, mainly include:
1. Individuals whose results from a complete physical examination do not meet the health standards, mainly include:
1) Abnormal vital signs with clinical significance (pulse < 50 times/min or > 100 times/min, systolic blood pressure >=140 mmHg or diastolic blood pressure >=90 mmHg on a conscious adult);
1. Complete uveitis remission: inactive disease for 3 months after discontinuing all treatments for eye disease.
2. Any known vaccine contraindications according to manufactures instructions:
(1) Individuals with a history of anaphylaxis to any component of the vaccine;
1. The subject has a history of SARS-CoV-2 infection, or has a history of contact with SARS-CoV-2 infected persons (nucleic acid test positive) or suspected infected persons within 30 days before screening, or living abroad within 30 days before screening history;
2. Anyone who is allergic to any vaccines, drugs, foods or known excipients

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Subjects who meet any of the following exclusion criteria should be excluded from the study:
1. Those who had history of COVID-19 or COVID-19 prophylactic drugs (e.g., any history of COVID-19 vaccine that has been listed or not);
2. Those who are tested positive for SARS-CoV-2 by etiological detection

1. Patients with severe cardiovascular, cerebrovascular, liver, kidney, malignant tumors, hematological diseases, and mental diseases;
2. Severe disturbance of consciousness, dementia or suffering from certain diseases affect the evaluation of curative effect, such as aphasia, visual and hearing impairment;

Exclusion Criteria:

included a physician decision that involvement in the trial was not in the patient's best interest, known allergic reaction and / or severe allergic to arbidol, blood system dysfunction (platelet count <100×10⁹/L, hemoglobin

Exclusion Criteria:

Severe type patients who comply with any of the following：

Shortness of breath, RR > 30 times/min;

1. Patients with other serious diseases such as lung diseases, cardiovascular and cerebrovascular diseases, hematopoietic diseases, autoimmune diseases, digestive system, or mental illness;
2. Pregnant or lactating women;
3. Participate in other forms of exercise during the trial

Exclusion Criteria:

have the medical history or family history of convulsion, epilepsy, encephalopathy and psychosis.
be allergic to any component of the research vaccines, or used to have a

1. Patients with severe cardiovascular, cerebrovascular, liver, kidney, malignant tumors, hematological diseases, and mental diseases;
2. Severe disturbance of consciousness, dementia or suffering from certain diseases affect the evaluation of curative effect, such as aphasia, visual and hearing impairment;

Exclusion Criteria:

have the medical history or family history of convulsion, epilepsy,encephalopathy and psychosis.
be allergic to any component of the research vaccines, or used to have a

Exclusion Criteria:

History of SARS-CoV-2 infection;
Any serious adverse reactions that are causally related to vaccination during the primary immunization of the previous inactivated SARS-CoV-2 vaccine;

Exclusion Criteria:

Having a history of COVID-19 or a positive nucleic acid test for COVID-19;
Having a history or a family history of allergy, convulsion, epilepsy, brain disease, or mental illness;

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Exclusion Criteria:

Sever Covid-19 Infection patients
SARS-Cov-2 specific antibodies (including IgM and IgG) were positive before included
Exclusion Criteria:

History of SARS-CoV-2 infection(laboratory confirmed);
Have received three and more doses of inactivated SARS-CoV-2 vaccine;
Severe adverse reactions, such as urticaria, dyspnea, and angioneurotic
Exclusion Criteria:

Suspected or confirmed fever within 72 hours before enrollment, or armpit temperature ≥37.3℃ on the day of enrollment;
Diastolic blood pressure >100mmHg and/or systolic blood pressure
1. Static heart rate > 100 beats/min; 2. Blood pressure < 90/60mmHg, > 140/90mmHg or blood pressure fluctuation more than 20mmHg, accompanied by obvious symptoms such as dizziness and headache; 3. Blood oxygen saturation <= 95%; 4. Combined with other diseases not suitable for exercise
1. Severe vomiting and difficulty in taking or ingestion of oral medication; 2. Pregnant and lactating women; 3. People with mental illness and cognitive impairment; 4. Subjects received antiviral drugs (including interferon-alpha, ribavirin, chloroquine phosphate, Abidol, remdesivir, lopinavir/ritonavir, monoclonal antibodies) in the week before admission; 5. Cases of respiratory
Exclusion criteria for first dose: 1) Within 14 days prior to vaccination, travel abroad and villages/communities affected by COVID-19, contact with novel coronavirus infected or suspected cases, persons in the quarantine period, or novel coronavirus infected or suspected cases in the same village/community; 2.) Novel coronavirus cases, suspected cases or
1. The patient's clinical data is incomplete.
2. The investigator believes that any serious concomitant systemic diseases, conditions or obstacles should be prevented from participating in this study: (1) Have a history of grade III-IV heart failure as defined by the New York Heart Association (NYHA), or a known left ventricular ejection fraction is
1.Known or suspected allergies to the components of azivudine tablets;
2.Women who are breast-feeding during pregnancy or have a family plan during the trial period and within 6 months after the end of the trial;
3.Participating in other clinical trials or using experimental drugs within 12 weeks before administration;
Those who meet any of the following criteria shall not be included in the group:
1. There were serious noninfectious pulmonary diseases, including pulmonary tumor, pulmonary edema, atelectasis, pulmonary embolism, pulmonary eosinophilic infiltration and pulmonary vasculitis;
Exclusion Criteria:

History of novel coronavirus infection confirmed or asymptomatic infected persons or positive nucleic acid test of novel Coronavirus;
SARS virus history;

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1.Medical history of COVID-19 or prior use of any medications other than 2 doses of inactivated vaccines to prevent COVID-19 (e.g., vaccination history of any other COVID-19 vaccines of other technical platform, marketed or not marketed, or vaccination history of more than 2 doses of inactivated Sinovac COVID-19 vaccine)

Exclusion Criteria:

History of contact with a SARS-CoV-2 infection (positive in nucleic acid test) within 14 days;

History of multiple system inflammatory syndrome (MIS-C);

The subjects will be excluded from the study for any of the following reasons:

1. SARS-CoV-2 PCR test is positive.
2. Those with a history of needle sickness;
3. Those with hypertension on day 0 before boost immunization (systolic blood pressure is >= 140mmHg and/or diastolic blood pressure is >=

1. Patients who have taken Xiyanping, Reduning, Tanreqing, Jinhua Qinggan, Banlangen and other antiviral Chinese medicines and/or immunomodulators, or drugs with immunomodulatory effects for external use within 1 week before being selected;

2. Currently suffering from diseases that seriously affect the immune system

Exclusion Criteria:

History of SARS-CoV-2 infection;

History of receiving COVID-19 vaccine;

History of asthma, history of allergy to the vaccine or vaccine components, or

Exclusion criteria for the first vaccination

Medical history or family history of convulsion, epilepsy, encephalopathy and psychosis.

HIV positive

Exclusion Criteria:

History of SARS-CoV-2 infection(laboratory confirmed);

Have received inactivated SARS-CoV-2 vaccine from other manufacturers other than Sinovac Life Sciences Co., Ltd, Beijing Institute of Biological

Exclusion Criteria:

History of SARS-CoV-2 infection(laboratory confirmed);

Have received three and more doses of inactivated COVID-19 vaccine;

Known allergy to vaccines or vaccine ingredients, and serious adverse

Main Exclusion Criteria:

Have oxygen saturation (SpO2) ≤93 % on room air at sea level or a ratio of arterial oxygen partial pressure (PaO2 in millimeters of mercury) to fractional inspired oxygen (FiO2) <300, respiratory rate >30 per minute, heart rate

1. Women who are in the menstrual period (day 1 to day 4 of menstrual cramps) or breastfeeding on the day of vaccination, or men whose partners plan to become pregnant within 12 months from the screening period to the full immunization period, or plan to donate sperm and eggs;

2. Those who have a history of new coronavirus vaccination;

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Exclusion Criteria:

Patients with past medical history or current clinically significant concomitant diseases;
History of malignancies within 5 years
Exclusion criteria:

Pregnancy or lactation period;
Infected with human immunodeficiency virus (HIV), hepatitis B virus (HBV) or hepatitis C virus (HCV);
Exclusion criteria:

Pregnancy or lactation period;
Infected with human immunodeficiency virus (HIV), hepatitis B virus (HBV) or hepatitis C virus (HCV);
1. other reasons caused short breath, sweating, dry cough, chest tightness, palpitations and other pulmonary heart function;
2. Patients with difficult drugs due to basic diseases;
3. with severe basic diseases that affect survival, including Uncontrolled has a clinical heart, lung, kidney, digestion, blood disease, neuroplastic disease
1) History of COVID-19, or positive results for either SARS-CoV-2 nucleic acid and antibody tests at screening (outside China only);
2) Suspected or confirmed fever (axillary temperature >=37.3 degree C/oral temperature >=37.5 degree C) within 72 hours prior to or on the day of enrollment;
The participant may not enter the trial if ANY of the following apply:
1.SARS-CoV-2 infection confirmed by RT-qPCR detection;
2.Clinical manifestations including fever, dry cough, fatigue, nasal congestion, runny nose, sore throat, myalgia, diarrhea, shortness of breath, dyspnea, etc.;
1. Medical history of COVID-19 or prior use of any medications other than 2 doses of inactivated vaccines to prevent COVID-19;
2. Abnormal laboratory test values are observed during screening period (exceed the upper or lower limit of the normal reference range by 1.2 times) and are considered clinically significant by the investigator (stage 1 subjects
Those who meet one of the following conditions will be removed: (1) patients with complications of other serious underlying diseases, such as chronic obstructive pulmonary disease, obstructive pulmonary disease, coronary heart disease, and hypertension; (2) patients with serious mental illness; (3) patients with cognitive dysfunction who are unable to understand the trial
1. SARS-CoV-2 PCR test is positive.
2. Those with an acute infection before the third vaccination.
3. Those with a history of needle sickness.
4. Those with a metal implant within 20cm around the planned injection site.
5. Those with a tattoo, keloid formation or hypertrophic scar at the planned
Exclusion Criteria:

Have the medical history or family history of convulsion, epilepsy, encephalopathy and psychosis.
Be allergic to any component of the research vaccines, or used to have a

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Exclusion Criteria:

HIV antibody positive;
HBsAg positive;
HCV antibody positive;

Exclusion Criteria:

RT-qPCR detects active SARS-Cov-2 infection;
a history of COVID-19, SARS, MERS infection (self-report, on-site inquiry);
a high-risk factor for COVID-19 infection;

Exclusion Criteria:

Presence of fever within 72 h before vaccination (axillary temperature ≥ 37.3℃), or active tuberculosis, or in the acute phase of other diseases;
A history of severe acute respiratory syndrome (SARS), Middle East

Exclusion Criteria:

Medical condition:

Known hypersensitivity to monoclonal antibody (mAb) or investigational

① Patients who are critically ill (invasive mechanical ventilation, severe heart failure or haemodynamic instability). ② Contraindications to prone positioning ((e.g. recent abdominal or thoracic surgery or trauma, facial, pelvic or spinal fractures, untreated pneumothorax, etc.). ③ Unable to cooperate due to severe mental illness, cognitive impairment, etc. ④

(1) Unable to complete the questionnaire; (2) Unable to tolerate getting out of bed.

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Count of Primary Outcomes
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Current Primary outcome measures

Clinical improvement time of 28 days after randomization
The 7-point scale
7 points: death
6 points: admission to ECMO and / or mechanical ventilation
5 points: Hospitalized for non-invasive ventilation and / or high-flow oxygen

Lower Murray lung injury score
Lower Murray lung injury score

The incidents of acute respiratory distress syndrome (ARDS) development

SOFA score

The time to 2019-nCoV RNA negativity in patients

The virological clearance rate of throat swabs, sputum, or lower respiratory tract secretions at day 7

Critically ill patients (%)

Number of participants with improvement from severe type to common type

The rate of virus inhibition

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Time to Clinical recoveryTime to Clinical Recovery (TTCR)

Time to Clinical Improvement (TTCI)

CT of lung
CT and MRI of hip

Clinical symptoms (fever, weakness, cough) recovery rate and recovery time

Clinical symptoms (fever, weakness, cough) recovery rate and recovery time

Viral nucleic acid detection
CT Scan of the Lungs
Blood routine examination
Temperature
Height
CURB-65
PSI score
Mechanical ventilation time
Length of stay in hospital

Antipyretic time
Time of virus turning negative

pulmonary function
St Georges respiratory questionnaire, SGRQ

pulmonary function
St Georges respiratory questionnaire, SGRQ
Modified Barthel Index, MBI
Incidence of adverse events

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pulmonary function
Antipyretic time
Time of virus turning negative

pulmonary function
St Georges respiratory questionnaire, SGRQ
Modified Barthel Index, MBI

Self-rating depression scale, SDS
Self-Rating Anxiety Scale, SAS
ocial support rate scale, SSRS

Rate of disease remission
Time for lung recovery

Rate of disease remission
Time for lung recovery

Rate of disease remission
Rate and time of entering the critical stage

novel coronavirus nucleic acid clearance rate

Relief of clinical symptoms and duration

Recovery time
Ratio and time for the general type to progress to heavy
Ratio and time of severe progression to critical or even death

The incidence of adverse outcome within 14 days after admission: Patients with conscious dyspnea, SpO2 = 94% or respiratory frequency = 24 times / min in the state of resting without oxygen inhalation

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Time to conversion of 2019-nCoV RNA result from RI sample

Time to viral negativity by RT-PCR
Time to clinical improvement

Time to viral negativityby RT-PCR
Time to clinical improvement: Time from start of study drug to hospital discharge or to NEWS2<2 for 24 hours.

Severe conversion rate
Oxygenation index
2019-nCoV nucleic acid test
Chest CT

The time when the nucleic acid of the novel coronavirus turns negative
T cell recovery time

PSI

Rate of comprehensive adverse outcome

Ventilation-free days

PSI

The time required for RNA Yin conversion of novel coronavirus in respiratory or blood specimens was compared between treatment groups The difference of

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Safety

Clinical symptom recovery rate

the negative conversion ratio
the proportion of general patients with advanced severe disease
confirmed rate of suspected cases
Incidence of onset at home / designated isolation population who progressed to designated isolation treatment or were diagnosed with novel coronavirus-infected pneumonia
days of onset at home / designated isolation population who progressed to designated isolation treatment or were diagnosed with novel coronavirus-

The incidence of composite adverse outcome within 14 days after admission: Defined as (one of them) SPO2<= 93% without oxygen supplementation, PaO2/FiO2 <= 300mmHg or RR <=30 breaths per minute.

Time to disease recovery

The virological clearance rate of throat swabs, sputum, or lower respiratory tract secretions at day 3
The virological clearance rate of throat swabs, sputum, or lower respiratory tract secretions at day 5
The virological clearance rate of throat swabs, sputum, or lower respiratory

Mortality in patients

disease remission
time to clinical improvement
overall rates of clinical improvement assessed at days 7, 14, and 28
time to chest CT improvement
time to virus nucleic acid negative-conversion
ECG
Chest imaging
Complications
vital signs
NEWS2 score

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the lowest SpO2 during intubation

oxygen index
max respiratory rate
lung radiography
count of lymphocyte
temperature
Length of stay
Length of severe
oxygenation index during treatment
all-cause mortality in 28 days
Peripheral blood cell count (including white blood cells, lymphocytes

Chest imaging (CT)

Chest CT
Routine blood test
liver and renal function
TCM syndrome

Validity observation index

vital signs (Body temperature, blood pressure, heart rate, breathing rate)
Respiratory symptoms and signs (Lung sounds, cough, sputum)
Etiology and laboratory testing
PaO2/SPO2
Liquid balance

the number of days between randomised grouping and clinical improvement

time to recovery.

Time to clinical recovery

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Negative conversion rate of 2019-nCoV nucleic acid
Lung inflammation absorption ratio

Negative conversion rate of COVID-19 nucleic acid
Lung inflammation absorption ratio

Rate of conversion to severe or critical illness

cure rate

Time to Clinical recovery

28-day survival
Inflammatory factor levels

Time to Clinical recovery

14 day outcome of the subjects, including: recovery, improvement, turning critical, death.
lung CT

Recovery time

Disappearance rate of fever symptoms

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Antipyretic time
Pharyngeal swab nucleic acid negative time
Blood gas analysis
Traditional Chinese medicine syndrome score

St. George's Respiratory Questionnaire

TCM symptoms efficacy

Improvement of clinical symptoms including duration of fever
Improvement of clinical symptoms including respiratory frequency
Number of participants with treatment-related adverse events evaluated with CTCAE,version 4.0

Number of patients who have progressed to suspected or confirmed within 24 days of exposure to new coronavirus

Proportion of patients with a lung injury score reduction of 1-point or more 7 days after randomization

Time of viral nucleic acid turns negative
Antipyretic time

rate of cure

Mortality rate

Pneumonia severity index
Oxygenation index (PaO2/FiO2)

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Time to clinical recovery after treatment
Rate of aggravation

Time of conversion to be negative of novel coronavirus nucleic acid

Length of admission
mortality rate

SOFA

time and rate of temperature return to normal
time and rate of improvement of respiratory symptoms and signs (lung rhones, cough, sputum, sore throat, etc.)
time and rate of improvement of diarrhea, myalgia, fatigue and other symptoms

TCM symptom score
Antifebrile time
The time and rate of transition of new coronavirus to Yin

Body temperature returns to normal time
Pulmonary inflammation resolution time (HRCT)
Mouthwash (pharyngeal swab) at the end of treatment COVID-19 RNA negative rate

Viral nucleic acid test

14 day outcome of the subjects, including: recovery, improvement, turning critical, death.
lung CT

the incidence of treatment failure in 14 days

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The improvement proportion of pulmonary fibrosis
The improvement of lung function

TTCI (Time to Clinical Improvement)

Time to Clinical Recovery, TTCR

Length of hospital stay

Number of worsening events

Lung function

Recovery time
Pneumonia psi score

Quality of life
Anxiety assessment
Depression assessment
Major symptoms improved
the rate of Coronary virus nucleic acid negative at 7 days, 14 days, 21 days, and 28 days after Ultra Short Wave Electrotherapy
systemic inflammatory response syndrome (SIRS) scores at 7 days, 14 days, 21 days, and 28 days after Ultra Short Wave Electrotherapy

Time to Clinical recovery
Butyrate in feces

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Length of hospital stay

Time to Clinical Recovery

Arterial Blood Oxygen Saturation
TTCR,Time to Clinical recovery
temperature
respiratory rate
Lung CT

Improved respiratory system function (blood oxygen saturation) recovery time

cough

Clinical recovery time
Clinical recovery time
Changes in viral load of upper and lower respiratory tract samples compared with the baseline

Main symptom relief time

lung function
ADL
6min walk

Time to Clinical recovery

cure rate

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nospital stay
Discharge rate
Site-specific hospital metastasis rate or severe conversion rate
Body temperature normalization time
Clinical symptoms disappearance rate

Recovery rate

Clinical symptoms
Blood routine
the viral load of 2019-nCOV of throat swab
TBNK cell subsets
TH1/TH2 Cytokine

Improvement of clinical symptoms (Clinical improvement is defined as a reduction of 2 points on the 6-point scale of the patient's admission status or discharge from the hospital)

viral negative-transforming time
30-day cause-specific mortality
30-day cause-adverse events
30-day all-cause mortality
co-infections

Mean clinical recovery time (hours)

Time to Clinical Recovery

MRC breathlessness scale
6MWD

COVID-19 nucleic acid detection time from positive to negative (respiratory secretion) or (3,5,7,10 days from positive to negative rate)
Lung CT observation of inflammation absorption
Proportion of cases with progressive disease
Neutrophil count
Lymphocyte count
Monocyte / macrophage count
Monocyte / macrophage function test
NK cell count

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Time of conversion to be negative of novel coronavirus nucleic acid

Proportion of COVID-19 close contacts who have developed as confirmed cases

Body temperature
TCM syndrome integral
Murray lung injury score
The transition time of novel coronavirus nucleic acid
MuLBSTA score

pneumonia severity index (PSI)
Incidence of new organ dysfunction

Time course of body temperature (fever)
Viral load over time

Clinical recovery time

Complete Blood Count
CRP
blood coagulation
D-dimer
Virological examination of pharyngeal swab

The days from positive to negative for viral nucleic acid testing

The time when the nucleic acid of the novel coronavirus turns negative
Conversion to heavy/critical type

the score of Hamilton depression scale
the score of Hamilton anxiety scale
the score of Self-Rating Depression Scale
the score of Self-Rating Anxiety Scale
the score of Athens Insomnia Scale

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The nucleic acid of the novel coronavirus is negative
CT scan of ground glass shadow disappeared

TTCI (Time to Clinical Improvement)

Chest imaging
RNA test of COVID-19
Time to remission/disappearance of primary symptoms: defined as the number of days when the three main symptoms of fever, cough, and shortness of breath are all relieved / disappeared

Time to leave ventilator on day 28 after receiving MSCs infusion

Clinical recovery time

Chest CT scan
Nucleic acid of novel coronavirus
Routine blood test
Routine urine test
Stool routine test

Ventilation-free days
mortality

Clinical index

CT scan of the lungs
Nucleic acid detection of throat secretion

CD8+ T cells numbers
CD4+ T cell numbers
NK cell numbers
Fatality rate
Clinical recovery time

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pulmonary function
Novel coronavirus pneumonic nucleic acid test

Cure rate
Mortality

Endotracheal intubation rate
mortality
Ratio of virus nucleic acid detection to negative

TCM symptom score
Becomes negative time of COVID-19
Cure / mortality rate

Change in lesion proportion (%) of full lung volume from baseline to day 28.

Size of lesion area by chest imaging
Blood oxygen saturation

pulmonary function

Routine physical examination
Vital signs: breathing, body temperature (armpit temperature)
Blood routine
C-reactive protein
Liver and kidney function test

Clinical recovery time
Pneumonia Severity Index (PSI) score

SPO2
lesions of lung CT
temperature
Blood routine
Inflammatory biomarkers

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Length of hospital stay
Discharge time of general type
Discharge time of severe type

Clinical recovery rate of day 7

Clearance rate and time of main symptoms (fever, fatigue, cough)

Rate of composite advers outcomes: SpO2, PaO2/FiO2, respiratory rate

NRS 2002 score
BMI
triceps skinfold thickness (TSF)
prealbumin
total albumin

Lung CT

Inflammation factor

The time to 2019-nCoV RNA negativity in patients

K-bld questionnaire survey
Refers to the pulse oxygen
chest CT
blood gas

During the operation, the patient's volume of local exhaled air from the mouth and nose, the patient's heart rate, respiratory rate and blood oxygen saturation

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Lung CT after 3 days
Lung CT after 7 days
Oxygenation parameters: SpO2, Partial arterial oxygen pressure (PaO2), PaO2/FiO2
Hospital stay

Status of immune function
The time of nucleic acid turns to negative
Length of stay in-hospital

Percent of subjects who have returned to room air
Percent change of subjects return to baseline oxygen requirement

Clinical symptom improvement rate: improvement rate of clinical symptoms = number of cases with clinical symptom improvement /number of enrolling cases * 100%

The percentage of patients who convert to moderate one
The rate of shock
Endotracheal intubation ratio
Time spent on the ventilator
mortality

Pharynx swabs, lower respiratory tract samples (sputum/endotracheal aspiration/alveolar lavage), and anal swabs rt-pcr of novel coronavirus nucleic acid negative conversion rate.

Temperature returns to normal for more than 3 days
Respiratory symptoms improved significantly
Pulmonary imaging showed that acute exudative lesions were significantly improved
Negative for two consecutive tests of respiratory pathogenic nucleic acid

Post-traumatic stress disorder checklist
Clinical prognostic outcome
Il-6
Health-related quality of life inventory

Psychological status
Treatment compliance
To evaluate the difference in isolation and rehabilitation of patients after discharge

PCL
PSQI
FSI

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SpO2 during intubation
the total time of intubation
Mask ventilation for SpO2 < 90%

Psychological status

fever clearance time
Effective rate of TCM symptoms

Progression rate from mild to critical/severe

Self-rating depression scale
self-rating anxiety scale
Athens Insomnia Scale

lasting time of fever
lasting time of novel coronavirus pneumonia virus nucleic acid detected by RT-PCR and negative result rate of the novel coronavirus disease nucleic acid

lasting time of fever
lasting time of novel coronavirus pneumonia virus nucleic acid detected by RT-PCR and negative result rate of the novel coronavirus disease nucleic acid

Incidence of side effects

The time of duration of COVID-19 nucleic acid RT-PCR test results of respiratory specimens (such as throat swabs) or blood specimens change to negative.

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PaO2 / FiO2 or respiratory rate (without oxygen)
Frequency of respiratory exacerbation
Observe physical signs and symptoms and record clinical recovery time
The number and range of lesions indicated by CT and X-ray of lung
Time for cough to become mild or absent

Survival rate

Time and rate of novel coronavirus become negative.

Real-time fluorescent RT-PCR detection of pharyngeal swab were used to detect the negative conversion rate of the new coronavirus nucleic acid (tested after the end of medication) and the negative conversion time(measured every other day after the start of medication, determined according to clinical needs)
Time to clinical improvement

Time to Clinical Recovery (TTCR)

Clinical therapeutic course
Pathogenic detectio
Chest CT
Laboratory indicators (blood routine, myocardial enzyme spectrum, inflammatory cytokines, etc.)
Chest imaging
Oxygenation index
Extravascular pulmonary water
pulmonary vascular permeability index, PVPI
Inflammatory factors

Recovery time
Pneumonia psi score

Pulmonary function
6 minutes walking distance
The Short Form -36 Healthy Survey
chest computerized tomography
Nucleic acid detection of novel coronavirus
TNF-alpha
IL-6
IL-10

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Tempreture
Virus nucleic acid detection

Clinical cure rate

cough
agitation

The time from randomization to clinical improvement

Time to Virus Eradication

Time to Virus Eradication

Time to clinical recovery after randomization

Lung CT

PO2/FiO2
ROX INDEX

Time to Clinical Recovery

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Medical imaging improvement rate in patients during recovery period

Shorten the duration of the disease
Antipyretic rate
Severe rate
Time of virus nucleic acid turning negative
Negative rate of viral nucleic acid
The time for the positive nucleic acid of the novel coronavirus to turn negative
Incidence of deterioration
The defervescence time
Chest CT
Primary symptom remission rate

blood gas
SOFA score

Cough Score
Expectoration score

changes in the ratio of PaO2 to FiO2 from baseline

HRCT pulmonary fibrosis score

serum albumin
siderophilin
prealbumin
lung CT scanning result

Albumin
Cellular immune indicators (total lymphocyte count (TLC) and T lymphocyte subsets (CD3, CD4))

COVID-19 nucleic acid negative conversion rate
Causal mortality
All-cause mortality

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Incidence of COVID-19 pneumonia
COVID-19 pneumonia rate of improvement

Improvement of clinical symptoms (Clinical improvement is defined as a reduction of 2 points on the 6-point scale of the patient's admission status or discharge from the hospital)

Maximal inspiratory pressure
Respiratory muscle evaluation

The change of TCM syndrome integral of deficiency of Fei-Pi-Qi-Xu Zhen
The results and changes of fatigue assessment scale at each time point

Changes of serum inflammatory factors
Patient death risk
Drug related adverse reactions and events

Time of Improvement or recovery of respiratory symptoms
Number of days virus nucleic acid shedding
Frequency of Improvement or recovery of respiratory symptoms

Inflammation absorption on Chest CT

insomnia
anxiety
depression
comfort

Pittsburgh sleep quality index
Self-rating anxiety scale
Self-rating depression scale
CD-RISC scale

HRCT score

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COVID-19 Nucleic acid

Changes of oxygenation index (PaO2/FiO2)
Side effects in the BM-MSCs treatment group

Clearance time of virus RNA

Recovery time

Clinical response
Side effects in the treatment group

Changes of oxygenation index (PaO2/FiO2) ,blood gas test

Hospital stay or death ratio of experimental group / death ratio of control group

Self-assessment of sleep quality
Efficient of sleep improvement

TTCI

FVC

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2019-ncov nucleic acid yin rate
Blood 2019-ncov nucleic acid positive
Positive 2019-ncov nucleic acid in feces
Nasopharyngeal swab positive for 2019-ncov nucleic acid

Viral nucleic acid test negative conversion rate

Changes in high-resolution computer tomography of the lung
Change in 6-minute walking distance

Exposure time in the medical cabin
Hospitalization period
Detection rate of arrhythmia

Lung CT Score

Occurrence of adverse reactions
Anti SARS-CoV-2 S IgG antibody response(ELISA)
Neutralizing antibody response to SARS-CoV-2

Monitoring of adverse events within 24 hours after infusion (including infusion related events and severe adverse reactions associated with non-primary disease)

Incidence of adverse reactions/events

Total distances of 6 minutes walk test

syndrome
nucleic acid of nCoV-19
blood count
liver and kidney function
cardiac enzymes

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Safety indexes of adverse reactions
Immunogenicity indexes of neutralizing-antibody seroconversion rates

the rate of 2019-nCoV RNA positive

SARS-CoV-2
SARS-CoV-2 IgM Antibody
SARS-CoV-2 IgG Antibody

Oxygenation index

mild cases: the critical illness rate of subjects at weeks 2
Severe or critical cases: the Changes of PaO2/FiO2 from baseline to day 1, 3, 5 and 24 hours later after the last administration
Safety data of COVID-19 patients treated with ulinastatin were collected

The proportion of responder at week 4

Six-minute Walk Test
lung CT

VAS

Visual analog score of symptoms

Virus negative time
Medication 3, 5, 7, 9, 11, 13, 15 days mouthwash (pharyngeal swab) 2019-nCOVRNA negative rate (%)

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clinical recovery time (TTCR)
the time of SARS-CoV2 overcasting

TCM syndrome curative effect

Improvement rate and time required for improvement (hours)
Clinical status decrease by two levels
Pulmonary-related mortality
All-cause mortality

Hospital stay
Modify Borg Dyspnea Score

N/A

ADL score
MMRC score

Incidence of adverse reactions/events

Detection of nucleic acid and antibody of new coronavirus

St George's respiratory questionnaire score
Fatigue assessment scale
Six minute walking experiment

Qualitative RT-PCR results of SARS-CoV-2 nucleic acid of oral / nasopharyngeal swabs, anal swabs, sputum and blood

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SARS-Cov-2 nucleic acid test

On day 7 and 14, Change (reduction) in viral load from baseline

Safety index-incidence of adverse reactions
Immunogenicity index-seroconversion rates of neutralizing antibody

Changes in diarrhea frequency and Bristol Stool Scale

Tolerance
Viral load of nCoV
Blood routine
Immunological examination
Blood liver and kidney function test

Changes in the degree of clinical symptoms such as fatigue, poor appetite, abdominal distension, and loose stools caused by decreased digestive function of the two groups of subjects

30-second sit-to-stand test score
treadmill test score
the 9-item Patient Health Questionnaire score
the 7-item Generalized Anxiety Disorder Scale score
the Revised Version of the Impact of Event Scale score

Time to disease recovery

Adverse reactions/events rate
Adverse reactions/events rate
Seroconversion rate of Neutralizing antibodies against SARS-CoV-2 Phase IIa (Day 0, 14 schedule)
Seroconversion rate of Neutralizing antibodies against SARS-CoV-2 Phase IIa

Appetite score

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Vital Capacity
Total Vital Capacity
Forced Vital Capacity
The Predicted Forced Vital Capacity
Diffusion Capacity for Carbon Monoxide

time to recovery

Modify Borg Dyspnea Score
Fatigue scale-14
Patient Health Questionnaire
Clinical Respiratory Symptom Scale

Throat Swab Nucleic Acid Test

Correlation of adverse events with the investigational product

Rate of aggravation

The number of adverse events after intramuscular injection

Psychological health level of the patient with COVID-19, assessed using the Chinese Version of Depression Anxiety and Stress Scale-21 (DASS-21) for all participants at baseline and post- intervention (1 day later)

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Neutralizing antibody positive conversion rate

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Adverse reactions/events rate
Adverse reactions/events rate
Seroconversion rate of Neutralizing antibodies against SARS-CoV-2 Phase IIb
Seroconversion rate of IgG antibodies against SARS-CoV-2 Phase Iib
Incidence of adverse events (AEs) by CTCAE v5.0
Proportion of subjects with SAEs
Proportion of subjects with infusion-related reactions
Proportion of subjects with hypersensitivity reactions
Incidence of adverse events (AEs) by CTCAE v5.0
Proportion of subjects with SAEs
Proportion of subjects with infusion-related reactions
Proportion of subjects with hypersensitivity reactions

Dose-limiting toxicity(DLT)
Maximal Tolerable Dose(MTD)

recovery rate of pulmonary function
recovery rate of renal function

Safety and tolerability of atomization (IFN- +TFF2) combined with single dose in healthy volunteers
Vital signs
Blood drug concentration

The number of adverse events after intramuscular injection
Immunogenic end point

Occurrence of solicited local reactions in the subjects (e.g., vaccination sites: pain/tenderness, erythema/redness, induration/swelling) during the 14-days after each dose of BNT162b1 or placebo.
Occurrence of solicited systematic reactions (e.g., nausea, vomiting, diarrhea, headache, fatigue, myalgia, arthralgia, chills, loss of appetite, malaise, and

Adverse Event
Serious Adverse Event

Occurrence of adverse reactions (AR) within 7 days after each dose of the recombinant SARS-CoV-2 vaccine (Sf9 cell) or placebo.

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Psychological status
Sleep status

Local and systemic adverse reactions/events within 14 days after each dose
All adverse reactions/events within 30 days after each dose
Serious adverse events (SAE) throughout the study period

Incidence of all adverse events related to vaccine (including incidence, severity, etc.)

Safety index-incidence of adverse reactions
Immunogenicity index-seroconversion rates of neutralizing antibody

Incidence of osteonecrosis of the femoral head

Incidence of osteonecrosis of the femoral head

Safety indexes of adverse reactions
Immunogencity indexes of GMT
Immunogencity indexes of neutralizing antibody

Efficacy Measurs(Time to negative conversion in viral nucleic acid test(by RT-PCR) for diagnostic samples)
Safety Measurements : 90 of participants with treatment-related adverse events as assessed byCTCAE v5.0

TCM syndrome score

Incidence of adverse reactions/events

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(VO2max)

Specific IgG antibody
Neutralizing antibody
Cellular immunity
Photo - C grade
SAS
SDS
PSQI
PSQI

The seropositive rates and level of SARS-CoV-2 neutralizing antibody
The seropositive rates of SARS-CoV-2 IgG antibody (tested by ELISA)

Immunogenicity index-geometric mean titer(GMT) of neutralizing antibody

Serum total antibodies, IgG antibodies, and neutralizing antibodies of SARS-CoV-2 one month after the last dose of vaccination (if the total antibodies against SARS-CoV-2 are positive, the positive conversion rate and antibody levels of IgG and neutralizing antibodies will be further tested)
The positive conversion rate and antibody level of SARS-CoV-2 total antibody

SARS-CoV-2 viral load

SARS-CoV-2 serum neutralizing titers - Seroconversion rates (SCR)
The geometric mean titer (GMT) of SARS-CoV-2 serum neutralizing titers at 1 month after dose 2

The proportion of adverse reactions (AR) up to Day 28 after prime and boost vaccination of the recombinant SARS-CoV-2 vaccine (CHO cell) or placebo.

Geometric mean (GMT) of specific antibody
The incidence of adverse reaction (AR)

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Adverse events
GMT of specific antibody against S protein of SARS-CoV-2
GMT of neutralizing antibody against S protein of SARS-CoV-2

SARS-CoV-2 neutralizing antibody
RBD protein binding antibody (IgG) detection.

Recovery time

Viral load
Clinical Symptoms (Fever, Cough...)
Inflammation
Pulmonary imaging
In-hospital days

Incidence of adverse events (AEs) by CTCAE v5.0
Proportion of subjects with SAEs

Serum neutralizing antibodies, IgG antibodies, and IgM antibodies of SARS-CoV-2 28 days after the last dose of vaccination

Serum neutralizing antibodies, IgG antibodies, and IgM antibodies of SARS-CoV-2 28 days after the last dose of vaccination
Serious adverse events (SAE) during the entire study period (6 months after the full vaccination
Serum neutralizing antibodies, IgG antibodies, and IgM antibodies of SARS-

Serum neutralizing antibodies, IgG antibodies, and IgM antibodies of SARS-CoV-2 28 days after the last dose of vaccination

IgG antibody
Neutralizing antibody

The primary endpoint is time to clinical improvement (censored at Day 28), defined as the time (in days) from randomization of study treatment (remdesivir or placebo) until a decline of two categories on a six-category ordinal scale of clinical status or live discharge

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Clinical symptoms (fever, cough,expectoration,chest tightness, polypnea, dyspnea) recovery rate and recovery time

The time and rate of nucleic acid turning negative during the isolation observation period

safety, tolerability, pharmacokinetic profile and immunogenicity
AE/SAE
Cmax
AUC0-inf
AUC0-t

The incidence of adverse reaction (AR)
The incidence of Adverse Events of Special Interest (AESI)

cough vas
mMRC
Borg score

Nucleic acid test positive rate of nasal and throat swabs during the isolation medical observation period

Hamilton Depression Scale

Incidence of adverse events (AEs)
Incidence of serious adverse events (SAEs)
Change from pre-dose baseline in RBC count
Change from pre-dose baseline in WBC count
Change from pre-dose baseline in Platelets count
neutralizing antibody
specific antibody
adverse events
Adverse reactions
blood biochemistry

Safety index-incidence of adverse reactions within 7 days after each dose
Immunogenicity index-seroconversion rates of neutralizing antibody against SARS-CoV-2

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The positive conversion rate of anti-SARS-CoV-2 specific neutralizing antibody (eucivirus neutralization assays)
The incidence of adverse reaction (AR)

duration of viral shedding
aggravation rate

Incidence of solicited adverse events within 7 days after vaccination.
GMT of neutralizing antibodies against live SARS-CoV-2 virus at Day 14 after the booster vaccination.

1. Positive conversion rate of serum anti-SARS-CoV-2 RBD protein antibody, and its geometric mean titer (GMT) and geometric mean fold increase (GMI)
2. Positive conversion rate of serum anti-SARS-CoV-2 neutralizing antibody, and its GMI

Safety

SARS-CoV-2 RBD specific antibody level

Incidence of Adverse Reactions (AR)
Incidence of Serious Adverse Events (SAE)

the incidence and severity of any adverse reactions/events within 30 minutes after each dose of vaccination
the incidence of abnormal blood biochemistry, blood routine, blood coagulation function and urine routine on the 4th day after each dose of vaccination
the incidence and severity of adverse reactions/events within 0-7 days after

The Geometric Mean Titer (GMT) of anti-SARS-CoV-2 neutralizing antibody
The four-fold increase rate of anti-SARS-CoV-2 neutralizing antibody

Security Index
Security Index

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Safety index-incidence of adverse reactions

Seroconversion rate
Antibody positive rate
Neutralizing antibody level

Antibody positive rate
Seroconversion rate
Neutralizing antibody level

Visual analogue scale for cough
modified Medical Research Council Dyspnea Scale (MMRC)
Borg perceived fatigue scale

changes in absolute lymphocyte counts

GMT of anti SARS-CoV-2 specific neutralizing antibody

Incidence of adverse reactions within 28 days after the booster dose.
GMT of neutralizing antibodies against live SARS-CoV-2 virus on day 14 after the booster dose.
The positive conversion rate and antibody level of COVID-19 serum IgG after second doses of 28d vaccination;
The positive conversion rate and antibody level of 28d IgA antibody in nasopharyngeal swabs after second doses of vaccination;
Levels of novel coronavirus spiking protein specific cellular immune response

adverse events;
adverse reactions;

The change of viral load;
Prognosis;

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GMT ratio of GMT-CLO-SCB-2019-010 over GMT-CLO-SCB-2019-003

Treatment-emergent Changes in Body Temperature
Treatment-emergent Changes in Respiratory Rate
Treatment-emergent Changes in Pulse Rate
Treatment-emergent Changes in Blood Pressure
Safety Laboratory Value (Haematology)

Seroconversion rate of the neutralizing antibody to SARS-CoV-2

The number of adverse events after intramuscular injection
Immunogenic end point

Incidence of solicited adverse events (AEs) after vaccination
Incidence of unsolicited AEs after vaccination

Geometric mean titer (GMT) of SARS-CoV-2 neutralising antibodies
Seroconversion rate (SCR) of SARS-CoV-2 neutralising antibodies
Geometric mean titer (GMT) of serum IgG antibodies
Seroconversion rate (SCR) of serum IgG antibodies
Geometric mean fold rise (GMFR) of SARS-CoV-2 neutralising antibodies
Immunogenicity index of GMT of the neutralizing antibody
Immunogenicity indexes of GMT of HI antibody for each of the 4 virus strains
Immunogenicity indexes of GMC of pneumococcal IgG antibody for the three serotypes (3, 19A and 19F)

Seroconversion rate of SARS-CoV-2 neutralizing antibody
Geometric mean titer of SARS-CoV-2 neutralizing antibody

Incidence of adverse reactions/events

Geometric mean titer(GMT) of neutralizing antibody to live SARS-CoV-2 in susceptible population

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Phase I clinical trial：The incidence of adverse reactions (ARs) .
Phase II clinical trial：The incidence of adverse reactions (ARs) .
Phase II clinical trial：The geometric mean titer(GMT) of specific antibody.
Phase II clinical trial：The geometric mean titer(GMT) of specific antibody.
Blood routine;
Blood biochemical;
Blood coagulation function;
Urine routines;
Urine pregnancy (for women of childbearing age only);

Immune five;
T lymphocyte subsets;
Pulmonary function (peak velocity meter measurement);

10 day rate of mild and general type conversed to severe type;

The change of viral load;

Rate of positive 2019-nCoV RNA test;

IgG antibody;
Neutralizing antibody as measured by pseudovirus neutralization assay;
Neutralizing antibody as measured by wild-type virus neutralization assay;

IgG antibody;
Neutralizing antibody;
Cellular immunity;

To compare the time to symptomatic worsening between prompt and deferred vaccination strategies for patients with uveitis;

The incidence of adverse events;
Occurrence of AE associated with vaccination;

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IgG antibody;
Neutralizing antibody;
Cellular immunity;
Cross neutralization;

fatigue scale;

Virus negative conversion rate in the first week

Recovery rate of clinical symptoms (fever, cough, expectoration, chest tightness, shortness of breath, dyspnea) and virus negative conversion rate (RT-PCR) at day 7

lung function;
ADL;
mMRC;
SF-36 scale;

Incidence of adverse reactions within 14 days after the booster dose.
GMT of neutralizing antibodies against live SARS-CoV-2 virus on day 14 after the booster dose.

fatigue scale;

Incidence of adverse reactions within 28 days after the booster dose.
GMT of neutralizing antibodies against live SARS-CoV-2 virus on day 14 after the booster dose.

Geometric mean titer(GMT) of neutralizing antibody to live SARS-CoV-2Safety index-incidence of adverse reactions

Seroconversion rate (COVAX)
Seroconversion rate (COVAX)
Neutralizing antibody level (COVAX)
Neutralizing antibody level (COVAX)
Neutralizing antibody level (COVAX)

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Clinical status at 28 days

Immunogenicity index-GMT of neutralizing antibodies (CZ02 strain)

Immunogenicity endpoint

cardio-pulmonary function;
ADL;
Lower limb function;
balanced capacity;

Time for viral nucleic acid to turn negative;

Humoral immunity (IgG, neutralizing antibody);

nucleic acid negative conversion time;
nucleic acid negative conversion rate;

time and rate of temperature return to normal;
time and rate of improvement of respiratory symptoms and signs (lung rhones, cough, sputum, sore throat, etc.);
time and rate of improvement of diarrhea, myalgia, fatigue and other symptoms;

Hyaluronic acid;
Virus negative time;

[Positive conversion rate of SARS-COV-2 neutralizing antibody]

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Neutralizing antibody as measured by wild-type virus neutralization assay;

Immunogenicity index-Seroconversion rate of the neutralizing antibody to live SARS-CoV-2
Immunogenicity index-Seroconversion rate of the neutralizing antibody to EV71

Adverse event;
GMT of binding antibody against S protein of SARS-CoV-2;
GMT of neutralizing antibody against S protein of SARS-CoV-2;

Inflammatory factors;
Immune cell genus;

Geometric mean titer(GMT) of neutralizing antibody to live SARS-CoV-2

Incidence of adverse reaction within 28 days after each dose
GMT of neutralizing antibodies against live SARS-CoV-2 virus on day 28 after the second dose.
Immunogenicity index-GMT of neutralizing antibodies
Immunogenicity index-seroconversion rate (4-fold and above increase)of neutralizing antibodies
Immunogenicity index-seropositive rate of neutralizing antibody

Immunogenicity index-GMT of neutralizing antibodies(CZ02 strain)

Number of treatment related Aes
Virologic efficacy Evaluation

IgG antibody;
Neutralizing antibody as measured by pseudovirus neutralization test;
Neutralizing antibody as measured by wild-type virus neutralization test;

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Safety: Incidence and severity of any adverse events (AEs) occurring during the clinical trial per CTCAE V5.0

The serum neutralizing antibody level of SARS-CoV-2.

The serum neutralizing antibody level of SARS-CoV-2.

Short breath
Hyperhidrosis
Chest tightness
Dry cough
Palpitation

Efficacy endpoint: Protective efficacy of symptomatic, virologically confirmed (RT-PCR positive) COVID-19 cases 28 days after full immunization.
Safety endpoint: The incidence of adverse events (AE) from the first vaccination to 30 days after full Immunization.

The incidence of effective immune reactivation on the 14th day of enrollment (Geometric mean titers (GMTs) of neutralizing antibodies produced in response to SARS-CoV-2 live virus increased 4 times or more from baseline, or changed from negative to positive from baseline)

Neutralizing antibody as measured by wild-type virus neutralization assay
Neutralizing antibody as measured by pseudovirus neutralization assay
IgG antibody

Resilience

Body Temperature
Adverse events
GMT of binding antibody against S protein of SARS-CoV-2
GMT of neutralizing antibody against S protein of SARS-CoV-2

Incidence of adverse reactions within 28 days after the booster dose.
GMT of neutralizing antibodies against live SARS-CoV-2 virus on day 28 after the booster dose in immunogenicity cohort.

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The incidence of adverse events (AE) and adverse reactions (ADR)
The incidence of grade 3-4 adverse events and adverse reactions
The incidence of serious adverse events (SAE).

Immunogenicity of Third Dose Vaccine Booster Following Two Doses of Inactivated Vaccines
Reactogenicity of Third Dose Vaccine Booster Following Two Doses of Inactivated Vaccines
Phase I: Incidence and severity of adverse reactions (ARS) from Day 0 to Day 7 days after each dose of vaccination.
Phase II: Geometric mean titers (GMT) and seroconversion rate of total IgG antibody (ELISA method) against the SARS-CoV-2 Alpha, Beta and Delta variants on Day 14 after the second dose of vaccination;
Incidence of adverse events (AEs)
Incidence of serious adverse events (SAEs)
Incidence of adverse event of special interests (AESIs)
Safety as determined by abnormality in haematology
Safety as determined by abnormality in clinical chemistry
rate of admission to ICU
Rate of tracheal intubation
change in supplemental oxygen delivery
chest CT

Quality of life

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Measure time point of outcome and Measure method of Current Primary outcome measures (If Specified)	Count of Secondary Outcomes
#BLANK	8
Lower Murray lung injury score [Time Frame: 7 days after randomization] Murray lung injury score decreased more than one point means better outcome.The Murray scoring system range from 0 to 4 according to the severity of the condition	8
The incidents of acute respiratory distress syndrome (ARDS) development [Time Frame: 14 days] The incidence rate of acute respiratory distress syndrome (ARDS) development	2
#BLANK	3
#BLANK	5
The virological clearance rate of throat swabs, sputum, or lower respiratory tract secretions at day 7 [Time Frame: 7 days after randomization]	5
#BLANK	3
Number of participants with improvement from severe type to common type [Time Frame: 2 weeks] Common type: Fever, respiratory tract and other symptoms, imaging examination shows pneumonia Severe type (meeting any of the following): (1) Respiratory	0
The rate of virus inhibition [Time Frame: Day 0, 2, 4, 7, 10, 14 and 21] Novel coronaviral nucleic acid is measured in nose / throat swab at each time point.	4

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Time to Clinical recovery Time to Clinical Recovery (TTCR) [Time Frame: up to 28 days] TTCR is defined as the time (in hours) from initiation of study treatment (active or placebo) until normalisation of fever, respiratory rate, and oxygen saturation, and alleviation of cough	10
Time to Clinical Improvement (TTCI) [Censored at Day 28] [Time Frame: up to 28 days] The primary endpoint is time to clinical improvement (censored at Day 28), defined as the time (in days) from randomization of study treatment (remdesivir or placebo) until a decline of two cate	10
#BLANK	0
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#BLANK	7
#BLANK	3
#BLANK	7
#BLANK	6
#BLANK	3
#BLANK	1

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#BLANK	5
#BLANK	1
#BLANK	0
Rate of disease remission [Time Frame: two weeks] A: For mild patients : fever, cough and other symptoms relieved with improved lung CT B:For severe patients : fever, cough and other symptoms relieved with improved lung CT SPO2> 93% or PaO2/FiO2>	5
Rate of disease remission [Time Frame: two weeks] A: For mild patients : fever, cough and other symptoms relieved with improved lung CT B:For severe patients : fever, cough and other symptoms relieved with improved lung CT SPO2> 93% or	5
Rate of disease remission [Time Frame: day 7] For mild patients: disease remission refers to relieved symptoms with improved lung CT For severe patients: disease remission refers to relieved symptoms with improved lung CT	5
novel coronavirus nucleic acid clearance rate [on day 6 after treatment; RT-PCR]	1
#BLANK	2
#BLANK	6
#BLANK	8

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#BLANK	6
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#BLANK	0
#BLANK	8
Rate of comprehensive adverse outcome [Time Frame: 14 days] The definition of comprehensive adverse outcome is as follows: SPO2≤93% without oxygen inhalation	9
Ventilation-free days [Time Frame: on the day 28 after enrollment] days without ventilation support during 28 days after patients' enrollment	8
#BLANK	8
#BLANK	8

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#BLANK	2
#BLANK	9
#BLANK	6
The virological clearance rate of throat swabs, sputum, or lower respiratory tract secretions at day 3 [Time Frame: 3 days after randomization]	2
The virological clearance rate of throat swabs, sputum, or lower respiratory tract secretions at day 5 [Time Frame: 5 days after	
#BLANK	7
#BLANK	4
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#BLANK	6
oxygen index [baseline, 1-week, 2-week, 3-week, 4-week] max respiratory rate [baseline, 1-week, 2-week, 3-week, 4-week] lung radiography count of lymphocyte [baseline, 1-week, 2-week, 3-week, 4-week] temperature [baseline, 1-week, 2-week, 3-week, 4-week]	3
Length of stay Length of severe oxygenation index during treatment[Day 1, 3, 5, 7, 10, 14, 21, 28] all-cause mortality in 28 days	1
#BLANK	3
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#BLANK	1
#BLANK	5
the number of days between randomised grouping and clinical improvement [within 28 days admission; Clinical improvement is defined as the patient's admission status of 6 grade scale score reduced by 2 points or discharged.]	6
time to recovery.[; clinical recovery was defined as sustained (48 hours) alleviation of illness based on symptom scores (fever, cough)]	8
#BLANK	11

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#BLANK	7
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#BLANK	6
#BLANK	4
Improvement of clinical symptoms including duration of fever [Time Frame: Measured from day 0 through day 28]	
Evaluation of pneumonia improvement	4
Improvement of clinical symptoms including respiratory frequency [Time Frame: Measured from day 0 through day 28]	
#BLANK	7
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Time of viral nucleic acid turns negative[After the subject's condition is significantly improved, two or more new coronavirus pneumonia virus nucleic acid tests (with an interval of more than 24 hours) are performed, and the test is negative for two consecutive times. The negative time is the time when the first	5
rate of cure[untile PCR negative; PCR]	3
#BLANK	7
Pneumonia severity index [Time Frame: From Baseline (0W) to 12 week after treatment]	
Evaluation of Pneumonia Improvement	8
Oxygenation index (PaO2/FiO2) [Time Frame: From Baseline (0W) to 12 week after treatment]	

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Time to clinical recovery after treatment [Time Frame: within 14 days from the start of medication] Defined as random to fever, respiratory rate return to normal and cough remission over 48 hours.	14
Rate of aggravation [Time Frame: within 14 days from the start	
#BLANK	1
#BLANK	2
#BLANK	3
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#BLANK	0
#BLANK	0
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the incidence of treatment failure in 14 days [Time Frame: 14 days] The clinical symptoms and signs continue to deteriorate, or new pulmonary or extrapulmonary lesions appear, or the chest imaging indicates the progress, and the patient is transferred to	4

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The improvement proportion of pulmonary fibrosis [Time Frame: Week 24] Evaluation of pulmonary fibrosis Improvement. pulmonary fibrosis judged by HRCT score.HRCT images are divided into four grades according to the score, and a reduction of one grade	6
TTCI (Time to Clinical Improvement)[within 28 days after randomization;]	6
Time to Clinical Recovery, TTCR[within 28 days after randomization;]	7
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Quality of life[; SF-12] Anxiety assessment[; SAS] Depression assessment [; SDS] Major symptoms improved	0
the rate of Coronary virus nucleic acid negative at 7 days, 14 days, 21 days, and 28 days after Ultra Short Wave Electrotherapy[Day 7, 14, 24, 28;]	2
systemic inflammatory response syndrome (SIRS) scores at 7 days, 14 days, 21 days, and 28 days after Ultra Short Wave	
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cough[evryday; Time of disappearance of cough]	4
Clinical recovery time[The twenty-eighth days; There are at least 2 improvements in clinical status grading (from discharge 1 to death 6) of 6 indicators in clinical evaluation]	5
Clinical recovery time[The twenty-eighth days; There are at least 2 improvements in clinical status grading (from discharge 1 to	
Main symptom relief time[; Temperature/oxygen saturation/respiratory rate]	9
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Mean clinical recovery time (hours) [Time Frame: up to 28 days]	
The clinical recovery time is defined as the time from initiation of study treatment (active or placebo) until normalisation of fever, respiratory rate, and oxygen saturation, and alleviation of cough	11
Time to Clinical Recovery[Observation until discharge or turn to severe; Duration of at least 72 hours from the start of study treatment to normal body temperature (underarm temperature <= 36.9 degrees C) and cough relief (evaluated by the cough symptom score scale) Use antipyretics, and in hours)]	10
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Time course of body temperature (fever) [Time Frame: 14 days]	
Compare the time course of body temperature (fever) between two groups over time.	25
Viral load over time [Time Frame: 14 days]	
Clinical recovery time[The twenty-eighth days;]	6
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Chest imaging[Before and after ozone treatment;] RNA test of COVID-19[Before and after ozone treatment;] Time to remission/disappearance of primary symptoms: defined as the number of days when the three main symptoms of fever, cough, and shortness of breath are all relieved / disappeared	6
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CT scan of the lungs[Every week; Double-lung CT examination] Nucleic acid detection of throat secretion[Every day; Pharyngeal secretion virus nucleic acid detection method]	6
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Endotracheal intubation rate[Within 14 days;] mortality[Within 30 days;] Ratio of virus nucleic acid detection to negative[14day;]	3
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Change in lesion proportion (%) of full lung volume from baseline to day 28. [Time Frame: Day 28] Evaluation of Pneumonia Improvement	19
Size of lesion area by chest imaging [Time Frame: At baseline, Day 1, Week 1, Week 2, Week 4, Week 8] Evaluation of Pneumonia change Blood oxygen saturation [Time Frame: At baseline, Day 1, Week 1, Week 2, Week 4, Week 8]	8
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Routine physical examination[Before entering the group, at the end of the sample;] Vital signs: breathing, body temperature (armpit temperature)[Before entering the group, at the end of the sample;]	0
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K-BIG questionnaire survey[4 weeks;] Refers to the pulse oxygen[4 weeks;] chest CT[4 weeks;] blood gas[4 weeks;]	7
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Status of immune function[everyday in-hospital; Flow cytometry or PCR]	
The time of nucleic acid turns to negative[everyday in-hospital; PCR]	1
Length of stay in-hospital[When out of the hospital; record]	
Percent of subjects who have returned to room air [Time Frame: 7 days]	
Percent of subjects who have returned to room air	0
Percent change of subjects return to baseline oxygen requirement [Time Frame: 7 days]	
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The percentage of patients who convert to moderate one[within 14day;]	
The rate of shock	10
Endotracheal intubation ratio	
Time spent on the ventilator	
Pharynx swabs, lower respiratory tract samples (sputum/endotracheal aspiration/alveolar lavage), and anal swabs rt-pcr of novel coronavirus nucleic acid negative conversion rate.[Day 15;]	7
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Psychological status[Enrollment and two weeks after enrollment; Chinese version of the depression-anxiety-stress scale (DASS-21)]	0
Treatment compliance[Enrollment and two weeks after enrollment; The patient's degree of cooperation with the	
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Psychological status[Enrollment and two weeks after enrollment; DASS-21]	2
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Survival rate [Time Frame: 12 weeks] Death event	1
Time and rate of novel coronavirus become negative.[Everyday; Nucleic acid test reagent]	3
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Clinical therapeutic course[; Fever, respiratory rate, blood oxygen saturation turned to normal and cough relieved for at least 72 hours. Chest CT showed that the inflammation was obviously absorbed, and the nucleic acid of respiratory pathogens was negative twice in a row (the sampling interval	2
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Clinical cure rate [Time Frame: 3 months] Definition of clinical cure: The viral load of the respiratory specimen was negative for two consecutive times (the interval between the two tests was greater than or equal to one day), the lung image improved, and the body temperature returned to	6
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The time from randomization to clinical improvement [Time Frame: No more than 28 days] The time from randomization to an improvement of two points on a seven-category ordinal scale or live discharge from the hospital	0
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Lung CT[Days 7 and 14;]	2
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changes in the ratio of PaO2 to FiO2 from baseline [Time Frame: Day 10]	14
HRCT pulmonary fibrosis score[Baseline, 7th day, 14th day, discharge period, 12th week;]	0
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COVID-19 nucleic acid negative conversion rate[12 week;] Causal mortality[12 week;] All-cause mortality[12 week;]	3

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Changes of serum inflammatory factors[; ELISA] Patient death risk Drug related adverse reactions and events	0
Time of improvement or recovery of respiratory symptoms [Time Frame: 10 days during the intervention period] Time of improvement or recovery of respiratory symptoms Number of days virus nucleic acid shedding [Time Frame: 10 days during the intervention period]	6
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insomnia[before intervention; after intervention or discharged; Insomnia severity index scale] anxiety[before intervention; after intervention or discharged; Self-rating anxiety scale] depression[before intervention; after intervention or discharged;	0
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HRCT score[Base line, week 1, 2, 4 after enrollment;]	2

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Changes of oxygenation index (PaO2/FiO2) [Time Frame: At baseline, 6 hour, Day 1, Day 3,Week 1, Week 2, Week 4, Month 6.] Evaluation of pneumonia improvement	7
Side effects in the BM-MSCs treatment group [Time Frame:	
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Recovery time [Time Frame: The day from the patient's admission to the Disease Progression to critical type，up to 2 weeks.] The time from the patient's admission to the Disease Progression to critical type，up to 2 weeks.	17
Clinical response [Time Frame: Up to 28 days] the efficacy of NKG2D-ACE2 CAR-NK cells in treating severe and critical 2019 new coronavirus (COVID-19) pneumonia	0
Side effects in the treatment group [Time Frame: Up to 28 days]	
Changes of oxygenation index (PaO2/FiO2) ,blood gas test [Time Frame: 12 months] Improvement of pulmonary function	4
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Self-assessment of sleep quality[; PSQI] Efficient of sleep improvement	5
TTCI [Time Frame: 1-28 days] Time to Clinical Improvement	9
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2019-ncov nucleic acid yin rate[; Endoscopic mucosal biopsy]	
Blood 2019-ncov nucleic acid positive Positive 2019-ncov nucleic acid in feces Nasopharyngeal swab positive for 2019-ncov nucleic acid	0
Viral nucleic acid test negative conversion rate [Time Frame: 5 months]	
Proportion of subjects who tested negative for nucleic acid from sputum or nasopharyngeal swabs for two consecutive times(sampling time at least 24 hours)	1
Changes in high-resolution computer tomograpny of the lung [Time Frame: 3 months]	
Changes in ground-glass shadows, interstitial or air nodules found on high-resolution computer tomography	4
Change in 6-minute walking distance [Time Frame: 3 months]	
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Occurrence of adverse reactions [Time Frame: 0-14 days post vaccination]	
Anti SARS-CoV-2 S IgG antibody response(ELISA) [Time Frame: 28 days post vaccination]	6
Neutralizing antibody response to SARS-CoV-2 [Time Frame:	
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Incidence of adverse reactions/events[0-7 days after each dose of vaccination;]	11
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Safety indexes of adverse reactions [Time Frame: From Day 0 to Day 28 after each dose] Incidence of adverse reactions occured from Day 0 to Day 28 after each dose	11
Immunogenicity indexes of neutralizing-antibody seroconversion	
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SARS-CoV-2[; Nucleic acid detection] SARS-CoV-2 IgM Antibody SARS-CoV-2 IgG Antibody	8
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The proportion of responder at week 4 [Time Frame: baseline, week 4] The responder is defined as the participants with ≥ 50% average VAS decrease of chest tightness and fatigue compared with baseline. The VAS ranges from 0 to 100	15
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clinical recovery time (TTCR) [Time Frame: not more than 14 days] TTCR is defined as the study treatment (oral aspirin enteric-coated tablet) began to fever, breathing rate, blood oxygen saturation recovery, and cough relieving for at least 72 hours	0
TCM syndrome curative effect[; TCM syndrome curative effect index]	5
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ADL score[discharge from the hospital;] MMRC score[discharge from the hospital;]	0
Incidence of adverse reactions/events[0-7 days after each dose of vaccination;]	7
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Safety index-incidence of adverse reactions [Time Frame: Day 0-28 after each dose vaccination]	
Incidence of adverse reactions after each dose vaccination	5
Immunogenicity index-seroconversion rates of neutralizing antibody [Time Frame: 28 days after the second dose	
Changes in diarrhea frequency and Bristol Stool Scale [Time Frame: daily, from date of randomization until the date of discharge or date of death from any cause, assessed up to 2 weeks.]	7
Including diarrhea in times/day, Bristol Stool Scale (the minimum	
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Adverse reactions/events rate [Time Frame: 7 days after vaccination]	
Occurence of adverse reactions/events after vaccination	13
Adverse reactions/events rate [Time Frame: 28 days after vaccination]	
Appetite score[On the third day of hospitalization and after 7 days intervention; Simplified Nutritional Appetite Questionnaire]	0

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Correlation of adverse events with the investigational product [Time Frame: 12 Weeks] Any adverse event, serious adverse event (SAE) occurred during the clinical study, including clinical symptoms and abnormal vital signs, abnormal laboratory tests (complete blood	4
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The number of adverse events after intramuscular injection [Time Frame: Up to one year after the last vaccination] The main observation methods of adverse reactions mainly include laboratory examination, local reactions and systemic reactions at the administration site, vital signs, blood routine	1
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Neutralizing antibody positive conversion rate [Time Frame: 30 days after inoculation] Neutralizing antibody positive conversion rate in the pre-immunization negative population 30 days after full vaccination	8

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Adverse reactions/events rate [Time Frame: 7 days after vaccination]	
Occurence of adverse reactions/events after vaccination	7
Adverse reactions/events rate [Time Frame: 28 days after vaccination]	
Incidence of adverse events (AEs) by CTCAE v5.0 [Time Frame: up to 24 weeks]	
Proportion of subjects with SAEs [Time Frame: up to 24 weeks]	1
Proportion of subjects with infusion-related reactions [Time Frame: up to 24 weeks]	
Incidence of adverse events (AEs) by CTCAE v5.0 [Time Frame: up to 24 weeks]	
Proportion of subjects with SAEs [Time Frame: up to 24 weeks]	1
Proportion of subjects with infusion-related reactions [Time Frame: up to 24 weeks]	
Dose-limiting toxicity(DLT) [Time Frame: 7 days] DLT will be assessed by DAIDS v2.1. The measurements include clinical symptoms and abnormal laboratory tests (complete blood cell count, serum chemistry, routine urinalysis, coagulation function, etc.) and 12-lead ECGs	6
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The number of adverse events after intramuscular injection [Time Frame: 12 months after full vaccination]	
The main observation methods of adverse reactions mainly include laboratory examination, local reactions and systemic reactions at the administration site	0
Occurrence of solicited local reactions in the subjects (e.g., vaccination sites: pain/tenderness, erythema/redness, induration/swelling) during the 14-days after each dose of BNT162b1 or placebo. [Time Frame: 14 days following each dose administration]	10
Adverse Event [Time Frame: 12 weeks] safety evaluation Serious Adverse Event [Time Frame: 12 weeks] safety evaluation	0
Occurrence of adverse reactions (AR) within 7 days after each dose of the recombinant SARS-CoV-2 vaccine (Sf9 cell) or placebo. [Time Frame: 7 days after each dose.]	12
Occurrence of solicited AR in the subjects within 7 days after each dose of recombinant SARS-CoV-2 vaccine (Sf9 cell) or	

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Safety index-incidence of adverse reactions [Time Frame: Day 0-28 after each dose vaccination] Incidence of adverse reactions after each dose vaccination. Immunogenicity index-seroconversion rates of neutralizing antibody [Time Frame: The 28th day after the second dose	4
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Safety indexes of adverse reactions [Time Frame: within 14 days post each vaccination] Occurrence of adverse reactions post vaccination	7
Immunogencity indexes of GMT [Time Frame: Day 28 post the second vaccination] Efficacy measurs(Time to negative conversion in viral nucleic acid test(by RT-PCR) for diagnostic samples) [Time Frame: 0-85days] Time to negative conversion in viral nucleic acid test (by RT-PCR) for diagnostic samples, negative conversion is defined as	9
TCM syndrome score[; Curative effect index of TCM syndrome]	4
Incidence of adverse reactions/events[0-28 days after each dose of vaccination;]	5

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(VO2max) [Time Frame: The change from baseline in maximum oxygen consumption (VO2max) at maximum exercise load at Week 12 of treatment.] The change from baseline in maximum oxygen consumption (VO2max) at maximum exercise load at Week 12 of treatment	32
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The seropositive rates and level of SARS-CoV-2 neutralizing antibody[Day 28 post full vaccination;] The seropositive rates of SARS-CoV-2 IgG antibody (tested by ELISA)[Day 28 post full vaccination;]	5
Immunogenicity index-geometric mean titer(GMT) of neutralizing antibody [Time Frame: Day 14 after the full course vaccination] Neutralizing antibody assay will be performed using the micro-neutralization method	4
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SARS-CoV-2 viral load [Time Frame: 7 Days] Change in time-weighted SARS-CoV-2 viral load in nasopharyngeal swabs from baseline to day 7	0
SARS-CoV-2 serum neutralizing titers - Seroconversion rates (SCR) [Time Frame: 1 Month after Dose 2] SCR of SARS-CoV-2 serum neutralizing titers at 1-month after dose 2. Seroconversion is defined as ≥4-fold rise from before vaccination to 1-month post dose 2	14
The proportion of adverse reactions (AR) up to Day 28 after prime and boost vaccination of the recombinant SARS-CoV-2 vaccine (CHO cell) or placebo. [Time Frame: 28 days after first dose]	7
Geometric mean (GMT) of specific antibody [Time Frame: 1 months after immunization in each study group] Geometric mean (GMT) of specific antibody against Severe Acute Respiratory Syndrome Coronavirus 2 (SARS-CoV-2) Spike protein Receptor-binding domain(S-RBD) protein (ELISA)	9

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Adverse events	
GMT of specific antibody against S protein of SARS-CoV-2 [30 days after the second dose of vaccine;]	0
GMT of neutralizing antibody against S protein of SARS-CoV-2 [30 days after the second dose of vaccine;]	
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Recovery time [Time Frame: From date of randomization until the date of discharge, assessed up to 6 months]	
The discharge criteria of Diagnosis and Treatment Protocol for Novel Coronavirus Pneumonia (Trial Version 7) are applied: a) body temperature is back to normal for more than three days	16
Viral load	
Clinical Symptoms (Fever, Cough...)[everyday; observation]	
Inflammation	4
Pulmonary imaging	
In-hospital days	
Incidence of adverse events (AEs) by CTCAE v5.0 [Time Frame: up to 24 weeks]	8
Proportion of subjects with SAEs [Time Frame: up to 24 weeks]	
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The incidence of adverse reaction (AR) [Time Frame: 0 to 7 days after each dose]	
The incidence of adverse reaction (AR)	9
The incidence of Adverse Events of Special Interest (AESI) [Time Frame: from day 0 to day 60 after last dose]	
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Incidence of adverse events (AEs) [Time Frame: Up to Day 181]	
Incidence of serious adverse events (SAEs) [Time Frame: Up to Day 181]	3
Change from pre-dose baseline in RBC count [Time Frame:	
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Safety index-incidence of adverse reactions within 7 days after each dose [Time Frame: Day 0-7 after each dose vaccination]	
Incidence of adverse reactions within 7 days after each dose	9
Immunogenicity index-seroconversion rates of neutralizing antibody against SARS-CoV-2 [Time Frame: The 28th day after	

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The positive conversion rate of anti-SARS-CoV-2 specific neutralizing antibody (eucivirus neutralization assays) [Time Frame: 30 days after full-course vaccination in each study group]	9
The incidence of adverse reaction (AR) [Time Frame: 0 to 7	
#BLANK	7
Incidence of solicited adverse events within 7 days after vaccination. [Time Frame: Within 7 days after vaccination]	
Incidence of solicited adverse events within 7 days after vaccination.	10
GMT of neutralizing antibodies against live SARS-CoV-2 virus at	
1. Positive conversion rate of serum anti-SARS-CoV-2 RBD protein antibody, and its geometric mean titer (GMT) and geometric mean fold increase (GMI)	2
2. Positive conversion rate of serum anti-SARS-CoV-2 neutralizing antibody, and its GMI [on 28 days after the last	
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SARS-CoV-2 RBD specific antibody level[baseline and 28 days after the second dose of vaccination; Chemiluminescence analysis,CLIA]	0
Incidence of Adverse Reactions (AR) [Time Frame: 0-14 days after each vaccination]	
The occurrence of Adverse Reactions (AR) in all groups	16
Incidence of Serious Adverse Events (SAE) [Time Frame: Within the first and final vaccination up until 12 months after the	
the incidence and severity of any adverse reactions/events within 30 minutes after each dose of vaccination [Time Frame: through 30 minutes after each dose]	11
Phase 1	
the incidence of abnormal blood biochemistry, blood routine	
The Geometric Mean Titer (GMT) of anti-SARS-CoV-2 neutralizing antibody [Time Frame: 14 days after the full course immunization]	7
Neutralizing antibody assay will be performed using the Microcytopathic assay	
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Safety index-incidence of adverse reactions [Time Frame: Day 0-28 after each dose] Incidence rate of adverse reactions occured from the beginning of the vaccination to 28 days after the second dose.	2
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changes in absolute lymphocyte counts [Time Frame: before and 48 hours after intervention] the changes in absolute lymphocyte counts (*10^9/L) in before and 48 hours after treatment	2
GMT of anti SARS-CoV-2 specific neutralizing antibody [Time Frame: 28 days after vaccination] GMT of anti SARS-CoV-2 specific neutralizing antibody in all groups (wild type variants test).	13
Incidence of adverse reactions within 28 days after the booster dose. [Time Frame: Within 28 days after the booster dose] Incidence of adverse reactions within 28 days after vaccination.	10
GMT of neutralizing antibodies against live SARS-CoV-2 virus	
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GMT ratio of GMT-CLO-SCB-2019-010 over GMT-CLO-SCB-2019-003 [Time Frame: Day 36]	
To demonstrate that SCB-2019 vaccine in this study of Chinese participants elicits immune response that is non-inferior to that in a randomly selected subset of participants of the CLO-SCB-	0
Treatment-emergent Changes in Body Temperature [Time Frame: up to 99 days]	
Grade of treatment-emergent changes in body temperature by Common Terminology Criteria for Adverse Event (CTCAE) scale version 5.0. The scale ranges from Grade 1 (Mild) to Grade 5	10
Seroconversion rate of the neutralizing antibody to SARS-CoV-2 [Time Frame: Day 28 after the second dose]	
Seroconversion rate of the neutralizing antibody to SARS-CoV-2 at day 28 after the second dose of inactivated COVID-19 vaccine	11
The number of adverse events after intramuscular injection [Time Frame: 12 months after full vaccination]	
The main observation methods of adverse reactions mainly include laboratory examination, local reactions and systemic reactions at the administration site	0
Incidence of solicited adverse events (AEs) after vaccination [Time Frame: 7 days after the first or second vaccination]	
Percentage of participants with solicited AEs (local, systemic) for 7 days following each primary vaccination (Days 0, 28) by intensity, relevance	6
Geometric mean titer (GMT) of SARS-CoV-2 neutralising antibodies [Time Frame: 56 days]	
Neutralizing antibody activity as detected by neutralization assay expressed as GMTs at multiple time points through Day 56	5
Immunogenicity index of GMT of the neutralizing antibody [Time Frame: Day 28 after the whole vaccination of COVID-19 vaccine]	14
GMT of the neutralizing antibody to live SARS-CoV-2 at day 28 after the whole vaccination of COVID-19 vaccine	
Geometric mean titer of SARS-CSeroconversion rate of SARS-CoV-2 neutralizing antibody [Time Frame: 28 days post full primary immunization course]	11
Geometric mean titer of SARS-CoV-2 neutralizing antibody [Time Frame: 28 days post full primary immunization course]oV-	
Incidence of adverse reactions/events [Time Frame: 0-28 days after each vaccination]	8
Geometric mean titer(GMT) of neutralizing antibody to live SARS-CoV-2 in susceptible population [Time Frame: Day 28 after the second dose]	12
GMT of the neutralizing antibody to live SARS-CoV-2 at day 28 after the second dose in susceptible population	

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Phase I clinical trial：The incidence of adverse reactions (ARs) . [Time Frame: Day 0-7 days after each vaccination.] Adverse reactions (ARs) in each dose group 0-7 days after each vaccination.	11
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Virus negative conversion rate in the first week [Time Frame: first week]	8
Recovery rate of clinical symptoms (fever, cough, expectoration, chest tightness, shortness of breath, dyspnea) and virus negative conversion rate (RT-PCR) at day 7 [Time Frame: Day 7]	23
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Incidence of adverse reactions within 14 days after the booster dose. [Time Frame: Within 14 days the booster dose]	8
Incidence of adverse reactions within 14 days after vaccination.	
GMT of neutralizing antibodies against live SARS-CoV-2 virus on day 14 after the booster dose. [Time Frame: On day 14 after	
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Incidence of adverse reactions within 28 days after the booster dose. [Time Frame: Within 28 days after the booster dose]	10
Incidence of adverse reactions within 28 days after vaccination.	
GMT of neutralizing antibodies against live SARS-CoV-2 virus on day 14 after the booster dose. [Time Frame: On day 14 after	
Geometric mean titer(GMT) of neutralizing antibody to live SARS-CoV-2 [Time Frame: 1 month after booster immunization]	3
GMT of the neutralizing antibody to live SARS-CoV-2 within 1month after booster immunization of the Inactivated SARS-CoV-	
Seroconversion rate (COVAX) [Time Frame: Results obtained 28 days after the first dose (= right before the second dose)]	1
the rate of positive seroconversion against coronavirus	
Seroconversion rate (COVAX) [Time Frame: Results obtained after the second dose of vaccination (= 56 days after the first	

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Clinical status at 28 days [Time Frame: At 28 days from inclusion]	
Clinical status at 28 days assessed using a six level ordinal scale，in this scale, 1 is the minimum score and presenting a better outcome as discharge, 2 means still in-hospital but no	4
Immunogenicity index-GMT of neutralizing antibodies（CZ02 strain） [Time Frame: Day 28 after booster vaccination] GMT of neutralizing antibodies（CZ02 strain） 28 days after booster vaccination.	6
Immunogenicity endpoint [Time Frame: 14 days after vaccination] GMT and positive rate of SARS-COV-2 neutralizing antibody 14 days after vaccination with recombinant Novel Coronavirus vaccine (CHO cells)	0
Baseline, 4 weeks, 8 weeks, 12 weeks [CPET] Baseline, 4 weeks, 8 weeks, 12 weeks [questionnaire survey] Baseline, 4 weeks, 8 weeks, 12 weeks [Lower Limb Section of the Simple Fugel-Meyer Rating Scale (FMA)] Baseline, 4 weeks, 8 weeks, 12 weeks [BBS]	0
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nucleic acid negative conversion time; nucleic acid negative conversion rate [3 days, 7 days, 10 days, and 14 days after vaccination injection, and 14 days and 28 days after discharge]	6
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Primary endpoint: [Time Frame: 14 days after vaccination] Positive conversion rate of SARS-COV-2 neutralizing antibody at day 14 after full inoculation of recombinant Novel Coronavirus vaccine (CHO cells).	5

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Immunogenicity index-Seroconversion rate of the neutralizing antibody to live SARS-CoV-2 [Time Frame: Day 28 after the second dose of COVID-19 vaccine]	9
Seroconversion rate of the neutralizing antibody to live SARS-CoV-2 at day 28 after the second dose of COVID-19 vaccine	
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Geometric mean titer(GMT) of neutralizing antibody to live SARS-CoV-2 [Time Frame: 28 days after the full immunization]	
Geometric mean titer(GMT) of neutralizing antibody to live SARS-CoV-2 28 days after the full immunization of each lot of COVID-19 vaccine	5
Incidence of adverse reaction within 28 days after each dose [Time Frame: within 28 days after each dose]	
Incidence of adverse reaction within 28 days after each dose.	8
GMT of neutralizing antibodies against live SARS-CoV-2 virus on day 28 after the second dose. [Time Frame: On day 28 after	
Immunogenicity index-GMT of neutralizing antibodies [Time Frame: 28 days after booster dose]	
GMT of neutralizing antibody 28 days after booster dose.	3
Immunogenicity index-seroconversion rate (4-fold and above increase)of neutralizing antibodies [Time Frame: 28 days after	
Immunogenicity index-GMT of neutralizing antibodies(CZ02 strain) [Time Frame: 14 days after the booster dose vaccination]	1
GMT of neutralizing antibodies(CZ02 strain) 14 days after the booster dose vaccination	
Number of treatment related AEs [Time Frame: 29 days after the last participant is randomized]	
Any AEs and SAEs occurring during the study	15
Virologic efficacy Evaluation [Time Frame: 7 days after the last	
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Safety: Incidence and severity of any adverse events (AEs) occurring during the clinical trial per CTCAE V5.0 [Time Frame: 85 days]	7
Incidence and severity of any adverse events (AEs) occurring during the clinical trial, including serious adverse events (SAEs)	
The serum neutralizing antibody level of SARS-CoV-2. [Time Frame: 8 weeks]	
The serum neutralizing antibody level of SARS-CoV-2 is determined by serum neutralization test, which is used to determine the serum dilution that protects 50% cells from	8
The serum neutralizing antibody level of SARS-CoV-2. [Time Frame: 8 weeks]	
The serum neutralizing antibody level of SARS-CoV-2 is determined by serum neutralization test, which is used to determine the serum dilution that protects 50% cells from	8
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Incidence of adverse reactions within 28 days after the booster dose. [Time Frame: Within 28 days the booster dose]	
Incidence of adverse reactions within 28 days after the booster dose.	11
GMT of neutralizing antibodies against live SARS-CoV-2 virus	

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The incidence of adverse events (AE) and adverse reactions (ADR) [Time Frame: 0-Day 28]	
Severity of adverse events was graded according to CTCAE5.0	2
The incidence of grade 3-4 adverse events and adverse reactions [Time Frame: 0-Day 28]	
Immunogenicity of Third Dose Vaccine Booster Following Two Doses of Inactivated Vaccines [Time Frame: Day 28]	
Observe the 28th day of the subjects' live virus neutralizing antibody geometric mean titers (GMTs) and mean geometric titers fold growth (GMFR) changes	4
Phase I: Incidence and severity of adverse reactions (ARs) from Day 0 to Day 7 days after each dose of vaccination. [Time Frame: From Day 0 to Day 7 after each dose]	15
Incidence and severity of adverse reactions occured from Day 0 to Day 7 after each dose of vaccination	
Incidence of adverse events (AEs) [Time Frame: From day1 to 6 months after administration.]	
To evaluate the safety and tolerability of a single dose of 600 mg AZD7442 administered IV to Chinese participants (including those with stable medical conditions) ≥ 18 years of age	0
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Current Secondary outcome measures [Measure time point of outcome Measure method]

The key secondary endpoint was time to clinical improvement (TTCI)
Proportion of patients in each category of the 7-point scale on days 7, 14, and 28 after randomization;
mortality on day 28
Duration of mechanical ventilation(days)
The difference of PaO2/FiO2 between two groups
Lower Sequential Organ Failure Assessment (SOFA) score
Mechanical ventilation support
The difference of PaO2/FiO2 between two groups
Lower Sequential Organ Failure Assessment (SOFA) score

The time to fever resolution rate
Time to recovery of lung injury

Duration of hospitalization
Proportion of mechanical ventilation use
Mortality
mortality on day 28
The rate of negative 2019-nCoV RNA results at day 14
the rate of aggravation
the rate of adverse events
The rate of discontinuations due to adverse events
The virological clearance rate of throat swabs, sputum, or lower respiratory tract secretions at day 3
The virological clearance rate of throat swabs, sputum, or lower respiratory tract secretions at day 5
Number of participants with treatment-related adverse events as assessed by CTCAE v5.0
Oxygenation index
PCT
Antibiotic use

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The disease prorogation-temperature
The disease prorogation-respiratory function 1
The disease prorogation-respiratory function 2
The disease prorogation-respiratory function 3

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All cause mortality
Frequency of respiratory progression
Time to defervescence (in those with fever at enrolment)
Time to cough reported as mild or absent (in those with cough at enrolment rated severe or moderate)
Clinical status
Time to Hospital Discharge OR NEWS2 (National Early Warning Score 2) of ≤ 2 maintained for 24 hours.
All cause mortality
Duration (days) of mechanical ventilation

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Single symptom disappearance rate and main symptom disappearance time
Proportion of aggravation during treatment (in line with the definition of severe or critical illness in the treatment scheme)
Rate of CT improvement
Disease recovery rate
Time and rate of coronavirus become negative
Proportion of aggravation during treatment (in line with the definition of severe or critical illness in the treatment scheme)
Rate of CT improvement
Single symptom disappearance rate and main symptom disappearance time

Hepatorenal function
Heart rate
Blood pressure

Antipyretic time
Pulmonary function
mortality and recovery rate
rate of multiple organ dysfunction
Time of virus turning negative
Pulmonary function
conversion rate of severe cases
Inflammation index
Disappearance time of gastrointestinal symptoms
Syndrome of traditional Chinese Medicine

Modified Barthel Index, MBI
Incidence of adverse events
6-minute walk test, 6MWT

6-minute walk test, 6MWT

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Disappearance time of cough
Incidence of adverse events
St Georges respiratory questionnaire, SGRQ
Modified Barthel Index, MBI
6-minute walk test, 6MWT

6-minute walk test, 6MWT

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Rate of no fever
Rate of respiratory symptom remission
Rate of lung imaging recovery
Rate of CRP,ES,Biochemical criterion (CK,ALT,Mb)recovery
Rate of undetectable viral RNA
Rate of no fever [Time Frame: two weeks]
Rate of respiratory symptom remission [Time Frame: two weeks]
Rate of lung imaging recovery [Time Frame: two weeks]
Rate of CRP,ES,Biochemical criterion(CK,ALT,Mb) recovery [Time Frame: two weeks]
Rate of undetectable viral RNA [Time Frame: two weeks]
Rate of normal tempreture
Rate of respiratory symptom remission
Rate of lung imaging recovery
Rate of laboratory indicator recovery
Rate of undetectable viral RNA

time till the SARS-CoV-2 clearance

Progression rate of 2019-nCoV pneumonia
Rate of severe pneumonia or death in people who progress to 2019-nCoV pneumonia

Improvement of TCM syndromes
Relief of clinical symptoms (fever, fatigue, gastrointestinal discomfort, etc.) and duration
Lung HRCT score improvement
Average length of hospital stay
Adverse event rate
Time from admission to clinical remission: Improvement of clinical symptom was defined as:
Patients with normal body temperature > 72 hours (without taking antipyretic drugs or hormones) and no conscious dyspnea or reduction of dyspnea
The proportion of patients with normal body temperature 14 days after admission
The proportion of patients without cough 14 days after admission

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all cause mortality
Time to temperature recovery
Time to improvement of respiratory symptom/signs
Length of hospitalization/ICU stay
Usage of oxygen therapy, mechanical ventilation:
ncidence of mechanical ventilation by day14
Incidence of ICU admission by day14
Time to treatment failure
All-cause mortality by day14, day28
The number (proportion) of subjects with viral positive by RT-PCR
ncidence of mechanical ventilation by day7, day14
Incidence of ICU admission by day7, day14
Time to treatment failure, determined as death, mechanical ventilation or ICU admission
All-cause mortality by day14, day28
Incidence of complications

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CT
X-ray
Arterial blood gas analysis
Assisted breathing use time
mortality
Time of clinical remission
Rate of no fever
Rate of no cough
Rate of no dyspnea
Rate of no need for oxygen inhalation
28-days mortality
ICU length of stay
Demand for first aid measuments
Vasopressor days
Respiratory indexes
CT
X-ray
Arterial blood gas analysis
Assisted breathing use time
mortality
Differences in mean clinical recovery time between treatment groups were compared
The proportion of subjects for the novel coronavirus RNA in respiratory or blood samples at day 7 of treatment was compared between treatment groups
RNA negative ratio of subjects for The novel coronavirus in respiratory or blood samples at day 14 of treatment was compared between treatment groups

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clinical symptom disappearance time
Time and rate of virus turning negative
Length of hospital stay
Antipyretic time
Severe conversion rate

Traditional Chinese medical syndrome scale
Body temperature

body temperature
Symptom Scale

Clinical recovery rate and time.
Rate of no fever within 14 days after admission.
Rate of no cough within 14 days after admission.
Rate of no dyspnea within 14 days after admission.
Rate of no recuring supplemental oxygen within 14 days after admission
Time and rate of coronavirus become negative
Clearance rate and time of main symptoms (fever, fatigue, cough)
Exacerbation (transfer to RICU) time
Routine blood test
Biochemical Indicators

Number of participants with treatment-related adverse events as assessed by CTCAE v5.0
The critical illness rate of subjects at weeks 2

Improvement rate in patients
Incidence of shock
Incidence of multiple organ failure
Days in hospital, days in ICU
Non-invasive ventilation modes and parameters
time to discharge
mortality
deterioration rate
adverse events

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the incidence of hypoxia
the times of intubation
Jaw thrust maneuver
Length of stay in ICU
Death

blood pressure
heart rate
immunoglobin level

Others (liver and kidney function, muscle enzymes, myoglobin)

Safty
Blood biochemistry index
viral nucleic acid fo 2019-nCoV

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Safety observation index

Hemodiafiltration
Mechanical ventilation time
Application of pulmonary surfactant
Oxygen time (hours)
ICU days
28-day mortality
Hospitalization time
ICU hospitalization
Invasive mechanical ventilation during infection
ECMO duration during infection
rate of composite advers outcomes.
rate of no fever (Time Frame: 14 days)
rate of no cough
rate of no dyspnea
rate of no requring supplemental oxygen
Time to defervescence
Time to absence of 2019-nCoV
Time to normalization of inflammation markers
Time to normalization of respiratory rate
Time to normalization of cough

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Lung inflammation absorption ratio
Time for body temperature to return to normal
Time of improvement in respiratory symptoms
Time to turn critical
Days in hospital
Time for body temperature to return to normal
Time of improvement in respiratory symptoms
Proportion of severe to light
All-cause mortality
Days in hospital
Rate of disisolation
TCM symptom
Body temperature
Blood routine test
Blood biochemical test
mortality
Ventilator utilization
Hospitalization day
Time to improvement of virus infection symptoms
Length of hospitalization /Healed and discharged
Time to conversion of 2019-nCoV RNA result from RI sample
Rate of preventing mild to moderate patients from shifting to severe patients
Changes in lung inflammation absorption

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Virus negative time and negative rate
Time and rate of disappearance of main symptoms (fever, fatigue, cough)
The time when the condition becomes worse (transfer to RICU)
BRT
Blood biochemistry
vital signs
oxygenation index
Routine Blood
inflammatory biomarkers
major organ function

COVID-19 RNA negative conversion rate
Disappearance rate of main symptoms(fatigue, dyspnea)

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Chest imaging
6-minute walking distance
Pulmonary function
EQ-5D
All-cause readmission
Conversion rate of common and critical illness
Nucleic acid negative time
Antipyretic time
The number of days in hospital
Time of virus nucleic acid test negative
CD4+ and CD8+ T cell count
Rate of mortality within 28-days
Size of lesion area by thoracic imaging
Clinical manifestations and severity of suspected or confirmed patients
Body temperature
Cough severity classification
Oxygenation index
Chest CT lesion size

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Arterial Oxygen Saturation
Critical (severe or critical) conversion rate
Inflammation index (CRP)
The disappearance rate and time of cough symptoms
Clinical recovery time
time to temperature normal
breath symptoms and measure
hospitalization days
Time of improvement of clinical symptoms
Remission time of fever
Duration of mechanical ventilation
Hospitalization time
Proportion of virus positive patients
Side effects in the UC-MSCs treatment group
28-days survival
Sequential organ failure assessment
C-reactive protein
Procalcitonin

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Clinical remission rate
Dynamic changes of oxygenation index
Time to cure
rate to cure
Time to defervescence

Remission time of fever

Tempreture
PSI score

cure rate
All-cause mortality
Severity rate

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Pneumonia symptoms
oxygenation index
Routine Blood
major organ function
Coagulation index
clinical cure incidence in 14 days
the duration of virus change to negative
mortality at day 30
ICU admission rate in 30 days

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The improvement proportion of pulmonary inflammation
The improvement proportion of clinical symptom
Quality of Life-BREF (QOL-BREF)
Patient Health Questionnaire-9(PHQ-9)
Generalized anxiety disorder-7(GAD-7)
Proportion of subjects with 6-grades rating form at fixed time points (days 7, 14, 21, and 28 after randomization)
All-cause mortality of 28-days
Duration of mechanical ventilation (days) of 28-days
Duration of supplemental oxygenation (days)
All-cause mortality of 28-days
Transition time of 2019-nCoV nucleic acid detection
Frequency of respiratory progression：Defined as SPO2≤ 94% on room air or PaO2/FiO2 <300mmHg and requirement for supplemental oxygen or more advanced ventilator support
Time to defervescence (days)
Length from the day of admission to the day returning to normal body temperature, from admission to two consecutive negative results of respiratory pathogenic nucleic acid test (sampling interval of at least 1 day), and from admission to the chest CT shows significant improvement.
The proportion of cases transformed from mild to severe illness
Total duration of disease
Average length of hospital stay
Time needed for healing
Negative rate of respiratory virus nucleic acid
Time from treatment to the beginning of antipyretic
Total duration of disease
Time needed for healing
Incidence of adverse events
Number of sequelae
SF-36
Clearance rate and time of main symptoms (fever, fatigue, cough、Gastrointestinal discomfort, chest tightness)
Clearance rate of single symptom
Exacerbation (transfer to RICU) time
Chest imaging score

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time to clinical recovey
the seven-category?ordinal scores at 7 days, 14 days, 21 days, and 28 days after Ultra Short Wave Electrotherapy
Improvement of gastrointestinal symptoms
Secondary bacterial infection rate
Improvement of chest CT
All cause mortality
Time to defervescence (in those with fever at enrolment)

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Vital signs
Respiratory symptoms
mental health
immune cells
All cause mortality
Length-of-stay in hospital
Length-of-stay in ICU
Days of mechanical ventilation
Modified RAP-65 and DECAF score
Frequency of exacerbations of dyspnea
Antipyretic time
Time to cough relief
Time to relieve dyspnea
The time when the 2019-ncov RT-PCR test of upper respiratory tract samples turned negative

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fever
feeble
novel coronavirus nucleic acid detection
Lung CT
Day 7, 14, 21, 28 clinical status
Time from study entry to discharge
Frequency of serious adverse drug reactions
Dynamic changes of laboratory indexes
Dynamic changes of daily vital signs (body temperature, respiratory rate, blood pressure, heart
Time to COVID-19 RNA convert negative
Other symptoms relief rate & time
Time & Rate of inflammatory biomarkers convert normal
Time to recovery
Frequency of antipyretic use
PHQ-9
Length of hospital stay
Respiratory symptoms
immune cells
Liver and renal function test
Antifebrile time
Inflammatory absorption of chest CT
Intervals from positive to negative of Nucleic acid test
Mortality rate
conversion rate of severe patients
mortality
Ventilator utilization
Hospitalization day

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Antifebrile time
Inflammatory absorption of chest CT
Intervals from positive to negative of Nucleic acid test
Time to Clinical recovery
Mortality rate

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Main clinical manifestations subsided or significantly improved (fever, dry cough, fatigue, etc.)
ICU hospitalization days
14 and 28-day all-cause mortality

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Time to CoVID-19 RT-PCR negative in upper respiratory tract specimen
Change (reduction) in CoVID-19 viral load in upper respiratory tract specimen as assessed by area under viral load curve.
Time to defervescence (in those with fever at enrolment)
Time to cough reported as mild or absent (in those with cough at enrolment rated severe or
The incidence of common Novel coronavirus pneumonia progressing to severe
Release and quarantine time
Time when common Novel coronavirus pneumonia progress to severe
Proportion to ICU
Mechanical ventilation rate

SF-36 score

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Remission time of fever

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28-day prognosis
SOFA
Mechanical ventilation time
Length of stay in ICU
Total hospital expenses
P/F ratio over time
Sequential organ failure assessment score(SOFA score) over time
Pulmonary Severity Index (PSI)
Image examination of chest over time
Proportion of subjects who progressed to critical illness or death
Time to 2019-nCoV R T-PCR negativity in upper and lower respiratory tract specimens
clinical status
Time from study entry to discharge
Frequency of serious adverse drug reactions
Dynamic changes of laboratory indexes

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Rate of progress to severe illness
Duration of fever
C reactive protein concentration in peripheral blood
Absolute value and percentage of lymphocytes in peripheral blood
Time of pulmonary imaging remission
T cell recovery time
Remission time of fever
Improvement of chest imaging
eukocyte and lymphocyte recovery time
Frequency of serious adverse drug reactions

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All-cause mortality
Time for body temperature to return to normal
Time of improvement in respiratory symptoms
Proportion of severe to light
Days in hospital
Conversion of severe cases: defined as the number of severe and critical cases that occurred after the study was included and the intervention was initiated
Incidence of medical complications during hospitalization: defined as medical complications during pneumonia isolation hospitalization due to novel coronavirus infection, including combined bacterial infections, exacerbation of underlying diseases, etc

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fever

Liver function test
renal function

ICU length of stay
Chest CT
Routine blood test
C-reactive protein
Liver and renal function

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Body temperature
3CL Mpro of Coronavirus
Routine blood test
Routine urine test
Liver function test
Arterial Oxygen Saturation
cytokines/chemokines
Hospital stay duration
Inflammation index (CRP)

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pulmonary CT
pulmonary CT
chest radiography

Length of stay

Inflammatory markers
Clinical symptoms
Liver and kidney function

Major symptom relief time
Blood gas analysis
Indicators of inflammation (CRP, ESR)
Chest CT
Change in lesion proportion (%) of full lung volume from baseline to day 10 and 90
Change in consolidation lesion proportion (%) of full lung volume from baseline to day 10, 28 and 90.
Change in ground-glass lesion proportion (%) of full lung volume from baseline to day 10, 28 and 90
Rate of mortality within 28-days
Sequential organ failure assessment
Side effects in the UC-MSCs treatment group
Electrocardiogram, the changes of ST-T interval mostly
Concentration of C-reactive protein C-reactive protein, immunoglobulin
6-minute walk test
basic ADL
IADL
anxiety
depression

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hepatic and renal function

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Conversion rate of severe type to general type
Conversion rate of general type to light type
Conversion rate of general type to severe type
Conversion rate of severe type to critical type
All-cause mortality during the trial
The rate of respiratory failure during the trial
The time from randomization to fever reduction
The time from randomization to cough relief
The time from randomization to dyspnea relief
Chest imaging
Refers to oxygen saturation
Respiratory rate
The rate and time at which the normal type progresses to the heavy type
Rate and time of progression from heavy to critical type and even death

novel coronavirus nucleic acid detection

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Nucleic acid detection of pharyngeal test
Leukocytes and lymphocytes in blood routine

7 day mortality
duration of mechanical ventilation
PD-1 expression of T cell
Absolute Lymphoblastic Counter
28 day mortality
The 7-point scale
The rate of critcal aggravation
Blood routine test outcomes
Blood biochemical test outcomes
Within 4 weeks of death from respiratory causes
Within 4 weeks of disease progression or death
Routine lymphocyte recovery
Changes in inflammatory markers of blood (il-8 et al.)
Absolute change in viral nucleic acid from baseline

New coronavirus infection before and after treatment, and new coronavirus infection among relevant medical staff (including chest CT, viral nucleic acid detection, clinical diagnosis, etc.)

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Sputum suction amount
Circulation indicators: blood pressure, heart rate

The incidence rate of complication

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The fading and obviously improving time of clinical symptoms (fever, dry cough, fatigue, etc)
The negative conversion rate of SARS-CoV-2 nucleic acid
Negative conversion time of SARS-CoV-2 nucleic acid
Abatement rate of fever transfusion .The definition of abatement rate refers to the case percentage with ≤ 37.5℃ axillary temperature and the maintenance of at least 48h
CRP
IL-6
TNF-α
Time of improvement of clinical symptoms (fever, cough)
Adverse drug reactions
1. 28-day clinical improvement time: the number of days from randomization to clinical improvement.Clinical improvement was defined as a reduction of 2 points on the 6-point scale of the patient's admission status or discharge.
2. Hospitalization duration of 28 days after admission: The number of days from randomization to discharge. Discharge criteria: body temperature returned to normal for more than 3 days;
Proportion of severe cases
Length of stay
Oxygen saturation
Mechanical ventilation time

Physical activity scale
Physical activity scale for the elderly,DISTRESS THEROMERTER

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the incidence of hypoxia
the times of intubation
7-day mortality

Treatment compliance
To evaluate the difference in isolation and rehabilitation of patients after discharge

body tempture
lung CT scan
clearance time of COVID-19
Oxygen saturation
duration of hospital stay
NEW Score
Hospital Stay
30-day all-cause mortality
Levels of Inflammatory Factors and Oxidative Stress Markersive Stress Markers

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Comparison of TCM symptom score (degree of fever, cough, sputum, chest tightness, shortness of breath, anorexia, abdominal distension, the tongue proper and the tongue coating, pulse condition)
comparison of changes in SpO2, oxygen saturation, lymphocyte count
Clinical recovery time: the body temperature returned to normal and lasting for more than 3
Comparison of TCM symptom score (degree of fever, cough, sputum, chest tightness, shortness of breath, anorexia, abdominal distension, the tongue proper and the tongue coating, pulse condition)
Comparison of changes in inflammation indicators (white blood cell count, C-reactive protein, ESR, procalcitonin) before and after treatment

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Time for improvement of clinical symptoms (eg fever, cough, dyspnea, etc.)
Imaging test improvement time (chest CT scan)
Occurrence of complications during hospitalization (such as severe sepsis, septic shock, and acute renal failure, etc.)
Mechanical ventilation time (days)

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body temperature

Level of IL2, IL4, IL6, IL10, TNF-alpha and IFN-gama
Routine blood test
lymphocyte subsets
Time to Clinical recovery
Proportion of cases with disease progression
Cough disappearance rate and time
All-cause mortality
Changes of cytokines (IL-2, IL-4, IL-6, IL-10, IL-17, INF-gama, TNF-alpha)
Time for temperature to return to normal
Time to 2019-nCoV RT-PCR negative in upper respiratory tract specimen
The rate of progression from mild/moderate type to heavy/critical type
4. Laboratory tests(white blood cell and lymphocyte count, erythrocyte sedimentation rate, C-reactive protein)

All-cause death
Adverse events

Mechanical ventilation time (days)
Total hospitalization time (days)
28-day all-cause mortality

Clearance rate and time of main symptoms (fever, fatigue, cough)
Disappearance rate of single symptom in traditional Chinese Medicine
Exacerbation (transfer to RICU) time
Chest imaging score

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Laboratory examination
Length of admission
Mortality rate
Incidence of adverse events in blood transfusion
Viral nucleic acid test negative conversion rate and days from positive to negative
Duration of fever
Lung imaging improvement time
Mortality rate because of Corona Virus Disease 2019
Rate of non-invasive or invasive mechanical ventilation when respiratory failure occurs

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The incidence of mild or common novel coronavirus pneumonia progressing to severe
Time for the main clinical manifestations to subside (fever, cough, respiratory rate, SPO2)

The incidence of mild or common novel coronavirus pneumonia progressing to severe
Time for the main clinical manifestations to subside (fever, cough, respiratory rate, SPO2)

28-day mortality
Hospitalization time
Incidence of breathing exacerbations
Time for conscious cough relief during infection (cough present when enrolled)
Time to remission of conscious dyspnea during infection (existed dyspnea upon enrollment)

Lung function
Arterial blood gas analysis

WBC
PBML
Time of Support time

All cause mortality
Frequency of exacerbation of dyspnea
Remission time of non-primary symptoms
Time for 2019-nCoV nucletic acid turning negtive in throat swabs
Duration of oxygen inhalation or noninvasive ventilation

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traditional Chinese medical syndrome scale
TCM symptom integral
TCM Constitution
body temperature
Serum antibody

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Haematological
C-reactive protein
Blood gas analysis
Urine test
Blood lipid
28 natural survival rate
TTCI (Time to Clinical Improvement)
Days in hospital
Duration of non-invasive / invasive ventilation
Proportion of severe to light

Hospital Stay
Inflammatory Factor Level

PaO2
blood oxygen saturation (SpO2)
clinical status rating on the 7-point ordinal scale
Time to Clinical Improvement (TTCI)
Duration (hours) of non-invasive mechanical ventilation or high-flow nasal catheter oxygen

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Length of hospital stay
Length from the day of admission to the day returning to normal body temperature, from admission to two consecutive negative results of respiratory pathogenic nucleic acid test (sampling interval of at least 1 day), and from admission to the chest CT shows significant improvement.
Recurrence rate of COVID-19.
Height
Weight
body mass index
upper arm circumference

Causal mortality
COVID-19 nucleic acid negative conversion rate
All-cause mortality

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Improving time of main clinical symptoms(wheezing, cough, sputum, etc)
ICU hospitalization days
14 and 28-day all-cause mortality

Dyspnea evaluation
6-Minute Walking Test
Activity of Daily Living
Evluation of International Classification of Functioning, Disability and Health
Hamilton Anxiety Scale

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Duration of fever
Frequencies of progression to severe illness
Time of improvement of pulmonary imaging
Peripheral blood c-reactive protein concentration
Absolute value of peripheral blood lymphocytes

Total hospitalized time
side effects associated with interventions

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Lung function
Oxygenation index

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Clinical outcome
Hospital stay
CT Scan
Changes in viral load
Changes of CD4+, CD8+ cells count and concentration of cytokines
Rate of progression to severe case
Duration of fever
Diagnosis value of C-reactive protein
Absolute value and percentage of peripheral blood lymphocytes
Lung image features improving time
Clinical Symptom Remission time
fever duration
Leicester cough questionaire (LCQ)
minimum oxygen
Negative conversion rate

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Detection of TNF-α levels, IL-10 levels
Detection of immune cells that secret cytokines, including CXCR3+, CD4+, CD8+, NK+ cells, and regulatory T cells (CD4 + CD25 + FOXP3 + Treg cells).
Changes of oxygenation index (PaO2/FiO2) ,blood gas test
Changes of c-reactive protein and calcitonin
SOFA
Serum inflammatory factor concentration
ICU inpatient days
Quality of life assessment in 90 day follow-up
Improving of PSQI
Improving of ISI
Improving of PHQ-9
Improving of GAD-7
Improving of SAS
Lung lesion
Immune function
Time of SARS-CoV-2 clearance
Blood test
SPO2
DLco%
6MWT
HRCT score
Change in cough

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Clinical cure rate

Changes in compound physiological index
Changes in the scores of the St. George's Hospital Respiratory Questionnaire
Changes in modified British Medical Research Council Dyspnea Scale (mMRC) scores
Changes in vital capacity of the lung

Total mortality
The incidence of sudden cardiac death

COVID-19 symptom score
Integral evaluation of qi deficiency and phlegm stasis syndrome
Pulmonary function assessment
St George's respiratory questionnaire score
Occurrence of adverse events
Occurrence of serious adverse reaction
Anti SARS-CoV-2 S IgG antibody response(ELISA)
Neutralizing antibody response to SARS-CoV-2
Neutralizing antibody response to Ad5-vector
Recovery time
Virus nucleic acid negative time and rate
The rate and time of main symptoms (fever, fatigue and cough) disappear
Pulmonary imaging changes
Arterial oxygen partial pressure and oxygenation index were measured 2 days after infusion
Incidence of abnormal indicators of liver and kidney function, blood routine and urine routine
Rate of adverse reaction and adverse event
Incidence of Serious Adverse Events (SAE)
Four-fold growth rate and antibody level (GMT, GMI) of serum antibody against COVID-19
Four-fold growth rate and antibody level (GMT, GMI) of COVID-19 antibody
pulmonary function
mMRC dyspnea scale
strength of lower limber muscles
ICF-COVID simple evaluation form
Quality of life

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Safety indexes of adverse reactions
Safety indexes of Incidence of abnormal laboratory index
Safety indexes of serious adverse events (SAEs)
Immunogenicity indexes of the seropositive rate, GMT, and GMI of neutralizing antibodies
Immunogenicity indexes of the seroconversion rate, seropositive rate, GMT and GMI of

body temperature
symptom

Covid-19 symptoms
TCM Constitution
Total IgG
Total IgA
IFN-α
Activity endurance
IL-6 level
Blood routine biochemical indicators
LUNG CT scan
microparticle
Lung CT imaging findings
Peripheral blood CD4+ cell count
blood lactic acid
oxygen saturation
The virological clearance rate of throat swabs, sputum, or lower respiratory tract secretions at
The proportion of responder at week 8
The change in the average VAS score of main symptoms from baseline
The change in the severity VAS score of main symptoms from baseline
The change in the average VAS score of other symptoms from baseline
The change in mean frequency of diarrhea and spontaneous bowel movements from baseline
Dynamic oxygen saturation
QQL-BREF
SARS-CoV-2 nucleic acid test and 2019-nCoV Antibody Test
PHQ-9
GAD-7

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Specific antibody IgM, IgG generation time
Changes in peripheral blood white blood cell counts, biochemical indicators, and blood coagulation from baseline on day 1, day 5, day 10, and day 15 after medication
Changes in C-reactive protein from baseline on day 1, day 5, day 10, and day 15 after medication

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Quality of Life
6MWD
Lung function
Chest CT examination
Reversion rate of viral nucleic acid
Percentage of patients need for non-invasive ventilation or HFNC
Percentage of patients need for invasive ventilation
Negative conversion time of SARS-COV-2 nucleic acid test
Time for relieving fever
Time for improvement of respiratory symptoms
Fatigue scale-14
Patient Health Questionnaire
Clinical Respiratory Symptom Scale
Vital signs

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Incidence of abnormal indicators of liver and kidney function, blood routine and urine routine
Incidence of adverse reactions/events
Incidence of Serious Adverse Events (SAE)
Four-fold growth rate and antibody level (GMT, GMI) of serum antibody against SARS-CoV-2
Four-fold growth rate and antibody level (GMT, GMI) of SARS-CoV-2 antibody

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2019-nCoV Antibody Test
Sars-cov-2 viral load levels in Oral / nasopharyngeal swab, anal swab, sputum, blood
Leukocyte, neutrophil, lymphocyte counts
lung CT
PBMC lymphocyte flow cytometry

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Fasting blood glucose
Liver function
Renal function
Electrolyte
Muscle enzyme
proportion of subjects change from mild or moderate type to severe type
proportion of subjects change from severe type to critical type
on days 7, 14 and 21, novel coronavirus nucleic acid conversion rate
Novel coronavirus nucleic acid negative conversion time
The time and proportion of improvement in pulmonary imaging
Safety index-incidence rate of adverse reactions
Safety index-incidence rate of serious adverse events in phase Ⅰ
Safety index-incidence rate of serious adverse events in phase Ⅱ
Immunogenicity index-seropositive rate, GMT, and GMI of neutralizing antibodies
Immunogenicity index-seroconversion rate, seropositive rate, GMT, and GMI in phase Ⅰ
IL-6 (ng/ml)
IL-10 (ng/ml)
IL-1β (ng/ml)
TNF-α (pg/ml)
leukocyte count (10^9/l)

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Time and rate of coronavirus become negative
Clearance rate and time of main symptoms (fever, fatigue, cough)
Exacerbation (transfer to RICU) time
Routine blood test
Biochemical Indicators
Serious adverse events
Level of Neutralizing antibodies against SARS-CoV-2 Phase Ia (Day 0, 14 schedule)
Level of Neutralizing antibodies against SARS-CoV-2 Phase IIa (Day 0, 14 schedule)
Level of IgG antibodies against SARS-CoV-2 Phase Ia (Day 0, 14 schedule)
Level of IgG antibodies against SARS-CoV-2 Phase IIa (Day 0, 14 schedule)

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Treatment-to-fever time (for patients with fever at admission)
Time between treatment and remission of cough (patients with moderate to severe cough when admitted)
Time from treatment to dyspnea relief (defined as mild or no dyspnea, applicable to patients with moderate dyspnea at admission)

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Primary pharmacokinetic variables
Primary pharmacokinetic variables
Primary pharmacokinetic variables
Primary pharmacokinetic variables
Clinical remission rate
Time and rate of laboratory indicators related to disease improvement to return to normal
Rate of patients with fever
Rate of patients with dyspnea
Rate of patients with requring supplemental oxygen

Immunogenic end point

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Specific IgG antibody
Neutralizing antibody
Cellular immunity
Neutralizing antibody GMT, positive rate
Neutralizing antibody GMT, positive conversion rate/positive rate
Neutralizing antibody GMT
Neutralizing antibody GMT
Neutralizing antibody GMT, positive rate

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Serious adverse events
Seroconversion rate of Neutralizing antibodies against SARS-CoV-2 Phase Ib
Seroconversion rate of IgG antibodies against SARS-CoV-2 Phase Ib
Level of Neutralizing antibodies against SARS-CoV-2 Phase Ib
Level of IgG antibodies against SARS-CoV-2 Phase Ib

Serum Concentration of BRII-196

Serum Concentration of BRII-198

AUC0-t
AUC0-∞
t1/2
Tmax
Anti-drug antibody(ADA)
six-minute walk test
pulmonary function recovery time
clinical symptoms
Lung CT recovery rate and recovery time
Novel coronavirus nucleic acid and antibody changes

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The proportion of subjects experiencing serious adverse events (SAEs), occurring up to Day 21 after prime vaccination and Day 28 after boost vaccination, up to Month 3, 6 and 12.
The proportion of subjects experiencing AE associated with BNT162b1, occurring up to Month 3, 6 and 12.
The proportion of subjects experiencing abnormal markers of hematology, blood chemistry and

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Occurrence of adverse events (AE) within 7 days after each dose of the recombinant SARS-CoV-2 vaccine (Sf9 cell) or placebo.
Occurrence of AE up to Day 28 after prime and boost vaccination.
The proportion of SAEs up to Day 28 after prime and boost vaccination.
The proportion of SAEs up to Month 12 after prime and boost vaccination

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Changes in laboratory indicators before vaccination and 3 days after vaccination (blood routine, blood biochemistry, coagulation function and urine routine)

Humoral immune response

Safety index-incidence of serious adverse events
Immunogenicity index-seropositive rates of neutralizing antibody
Immunogenicity index-geometric mean titer (GMT) of neutralizing antibody
Immunogenicity index-geometric mean ratio (GMR) of neutralizing antibody
TCM syndrome curative effect index
Evaluation criteria of TCM for osteonecrosis of the femoral head
General physical signs
Electrocardiogram
Laboratory physical and chemical indicators
TCM curative effect index
Evaluation criteria of TCM for osteonecrosis of the femoral head
General physical signs
Electrocardiogram
Laboratory physical and chemical indicators
Safety indexes of adverse events
Safety indexes of Hematological examination measures (Hemoglobin, WBC)
Safety indexes of Blood routine measures(ALT, AST)
Safety indexes of SAE
Immunogencity indexes of GMT
PK Measures:AUC0-tau
PK Measures：Cmax
PK Measures：Tmax
PK Measures：t1/2
PK Measures：Cl
Summary of the World Health Organization quality of Life scale
Score of main symptoms and signs
Pulmonary function
Chest CT examination
Serious Adverse Events(SAE)
Abnormal indicators of blood routine, blood biochemistry, urine routine and coagulation function
The seropositive rates and level of SARS-CoV-2 neutralizing antibody and IgG antibody(tested by ELISA)

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(VO2max)
(VO2max)
(VE /VCO2)
(VE /VCO2)
(VE /VCO2)

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Incidence of adverse reactions/events
Serious Adverse Events (SAE)
Level of SARS-CoV-2 IgG antibody (tested by ELISA)
The seropositive rates and level of SARS-CoV-2 neutralizing antibody and IgG antibody (tested by ELISA)
Immunogenicity index-Seroconversion rate of the neutralizing antibody
Immunogenicity index-Seropositive rate of the neutralizing antibody
Safety index-Incidence of adverse reactions
Safety index-Incidence of serious adverse events
Local and systemic adverse reactions/events within 14 days after each dose
All adverse reactions/events within 42 days after each dose
Serious adverse events (SAE) during the entire study period (12 months after the full vaccination)

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SARS-CoV-2 serum neutralizing titers - SCR
SARS-CoV-2 serum neutralizing titers - GMT
SARS-CoV-2 anti-S1 immunoglobulin G (IgG) antibody level - SCR
SARS-CoV-2 anti-S1 IgG antibody level - GMT
SARS-CoV-2 serum neutralizing antibody level - Geometric mean fold rise (GMFR)
The proportion of adverse events (AE) within 7 days after each dose of the recombinant SARS-CoV-2 vaccine (CHO cell) or placebo.
The proportion of abnormal markers of hematology, blood chemistry and urine analysis within 3 days after each dose of the recombinant SARS-CoV-2 vaccine (CHO cell) or placebo
The proportion of serious adverse events up to Month 12 after prime and boost vaccination
The incidence of adverse events (AE)
The incidence of severe adverse events (SAE)
The incidence of serious adverse events
Geometric mean (GMT) of anti-SARS-CoV-2-specific neutralizing antibodies
The positive conversion rate of S-RBD protein-specific antibody

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Time of disappearance of fever symptoms
The rate of conversion from COVID-19 positive to COVID-19 negative
Time of disappearance of cough
Respiratory rate
Blood oxygen saturation
routine blood test
blood biochemical
routine urine test
Blood oxygen
Proportion of subjects with infusion-related reactions
Proportion of subjects with hypersensitivity reactions
Mean of change from pre-dose baseline in ECG readings
Mean of change from pre-dose baseline in WBC count.
Mean of change from pre-dose baseline in RBC count
The specific immune cell level of SARS-CoV-2 in whole blood 28 days after the last dose of vaccination
Local and systemic adverse reactions/events within 7 days and 6 months after each dose
Serious adverse events (SAE) during the entire study period (6 months after the full vaccination
Serum neutralizing antibodies, IgG antibodies, and IgM antibodies of SARS-CoV-2 after enhancing immunity of vaccination
The specific immune cell level of SARS-CoV-2 in whole blood 28 days after the last dose of vaccination
Local and systemic adverse reactions/events within 7 days and 6 months after each dose
The specific immune cell level of SARS-CoV-2 in whole blood 28 days after the last dose of vaccination
Serum neutralizing antibodies, IgG antibodies, and IgM antibodies of SARS-CoV-2 after enhancing immunity of vaccination
Local and systemic adverse reactions/events within 7 days and 6 months after each dose

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Improvement time of clinical symptoms(disappearance of pulmonary rales, recovery of body temperature, absorption of pulmonary lesions> 50%, recovery of blood picture)
Changes of oxygenation index in blood gas analysis (up to 28 days)
To turn a standard time in the ICU(up to 28 days)
Duration (days) of mechanical ventilation (up to 28 days)

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Single symptom disappearance rate and main symptom disappearance time
Changes in color, quality and quantity of sputum in patients with expectoration
Changes in oxygenation index (OI)
Proportion of aggravation during treatment (in line with the definition of severe or critical illness in the treatment scheme)
Clinical symptoms and severity during the isolation observation period
Clinical symptoms appearance time and proportion during the isolation observation period
Proportion of mild and common cases of novel coronavirus pneumonia diagnosed during the isolation observation period

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The incidence of adverse events (AE)
The incidence of grade 3 adverse events (AE)
The incidence of severe adverse events (SAE)
Geometric mean (GMT) of specific antibody
Geometric mean (GMT) of anti-SARS-CoV-2-specific neutralizing antibodies
TCM syndrom score
PHQ-9 score
GAD-7 score
routine blood test
Time of occurrence of positive nucleic acid tests in nasal and throat swabs during the isolation medical observation period
Proportion of asymptomatic infections among positive nucleic acid tests
Proportion of mild, normal, and severe patients with positive nucleic acid tests
Date of onset during the isolation medical observation period (that is, the time when the clinical
Hamilton Anxiety Scale
Patient Health Questionnaire-15
Scale of severity of insomnia
Adverse Reaction Scale
Duration of COVID-19 related symptoms through Day 29 among subjects with mild-moderate COVID-19
Proportion of subjects who have mild-moderate COVID-19 and develop severe COVID-19 after randomization
Assessment of PK parameters (Group A and B): maximum serum concentration observed

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Safety index-incidence of adverse reactions within 56 days after the first dose vaccination
Safety index-incidence of serious adverse events
Immunogenicity index-seropositive rates of neutralizing antibody against SARS-CoV-2
Immunogenicity index-geometric mean titer (GMT) of neutralizing antibody against SARS-CoV-2

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The incidence of adverse events (AE)
The incidence of severe adverse events (SAE)
The Geometric mean titer (GMT) of anti-SARS-CoV-2 specific neutralizing antibody (euvirus and pseudovirus neutralization assays)
The Geometric mean fold increase (GMI) of anti-SARS-CoV-2 specific neutralizing antibody
Routine blood
Biomedical indexes
Blood biochemical
The length of time
Imaging CT changes
Incidence of adverse reactions within 28 days after vaccination.
Incidence of adverse events within 28 days after vaccination.
Incidence of unsolicited AE within 28 days after vaccination.
Incidence of serious adverse events(SAE) from the first dose to the 6 months after completing the last dose of vaccination

Safety outcomes
Immunogenicity outcomes

Immunogenicity

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Incidence of Adverse Events/Reactions (AE/AR)
Incidence of Adverse Events/Reactions (AE/AR)
Changes in laboratory test indicators
GMT of Covid-19 S protein RBD specific antibody
Seroconversion rate of Covid-19 S protein RBD specific antibody
Geometric Mean Titer (GMT) of SARS-CoV-2 Specific Neutralizing Antibody (Live SARS-CoV-2 neutralization assay) On the 30th day after the full course of vaccination, the neutralizing antibody level (wild Strains)
Seroconversion and Geometric Mean Titer (GMT) of SARS-CoV-2-Specific Binding Antibody (IgG) Before each dose, the 15th day after the second dose, the 15th, 30th, 90th, 180th, 360th
The Geometric Mean Titer (GMT) of anti-SARS-CoV-2 neutralizing antibody
The four-fold increase rate of anti-SARS-CoV-2 neutralizing antibody
The Geometric Mean Titer (GMT) of anti-SARS-CoV-2 neutralizing antibody
The four-fold increase rate of anti-SARS-CoV-2 neutralizing antibody
Immune Persistence

immunogenicity index
Immunogenicity indicators

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Safety index-incidence of adverse reactions
Safety index-incidence of serious adverse events

Adverse reaction

Adverse events following vaccination

TCM syndrome score
Depression Scale (PHQ-9), anxiety scale (GAD-7)
Blood oxygen saturation
6 minutes walking distance
Chest CT

the death in hospital
the composite endpoint of aggravation

Seroconversion rate of SARS-CoV-2 neutralizing antibody
GMI of SARS-CoV-2 neutralizing antibody
GMT of SARS-CoV-2 S protein RBD antibody
Seroconversion rate of SARS-CoV-2 S protein RBD antibody
GMI of SARS-CoV-2 S protein RBD antibody
Incidence of solicited AE within 14 days after the booster dose
Incidence of unsolicited AE within 28 days after the booster dose.
Incidence of serious adverse events (SAE) till the 6 months after the booster dose.
GMT of binding antibodies against SARS-CoV-2 S and N protein on day 14, day 28 and month 6 after the booster dose
Local and systemic adverse reactions / events within 14 days after each dose
All adverse reactions / events within 42 days after each dose
Severe adverse events (SAE) during the whole study period (12 m after the whole course of vaccination)
Virus specific antibody
Virus neutralizing antibody
Indicators of blood infectious diseases
Blood routine test
Blood biochemical index
Nucleic acid negative time
The ratio of nucleic acid to negative
Major symptoms disappeared
CT imaging changes of chest
Hospital stays

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Assess Pharmacokinetic parameters of LY-CovMab
Assess Pharmacokinetic parameters of LY-CovMab
Assess Pharmacokinetic parameters of LY-CovMab
Assess Pharmacokinetic parameters of LY-CovMab
Assess Pharmacokinetic parameters of LY-CovMab
Seroconversion rate of the neutralizing antibody to SARS-CoV-2
Seropositivity rate of the neutralizing antibody to SARS-CoV-2
GMT of the neutralizing antibody to SARS-CoV-2
GMI of the neutralizing antibody to SARS-CoV-2
Geometric mean concentrations of anti-pneumococcal antibody for 23 serotypes (ELISA)

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Incidence of serious AEs (SAEs) and adverse events of special interest (AESIs)
Proportion of subjects with abnormal markers of hematology, biochemistry, urinalysis and coagulation parameters
Geometric mean titer (GMT) of SARS-CoV-2 neutralising antibodies
Seroconversion rate (SCR) of SARS-CoV-2 neutralising antibodies
Percentage of participants reporting adverse events (AEs)
Percentage of participants reporting solicited Aes
Percentage of participants reporting unsolicited Aes
Percentage of participants reporting serious adverse events (SAEs)
Percentage of participants reporting adverse events of special interest (AESIs)
Immunogenicity index of seroconversion rate of the neutralizing antibody
Immunogenicity index of seropositivity rate of the neutralizing antibody
Immunogenicity index of GMI of the neutralizing antibody
Immunogenicity indexes of seroconversion rate of HI antibody for each of the 4 virus strains
Immunogenicity indexes of seroprotection rate of HI antibody for each of the 4 virus strains
Incidence of adverse reactions/events
Seroconversion rate of SARS-CoV-2 IgG binding antibody
Geometric mean titer of SARS-CoV-2 IgG binding antibody
Geometric mean titer of SARS-CoV-2 IgG binding antibody
Geometric mean titer of SARS-CoV-2 neutralizing antibody
Incidence of Serious Adverse Events (SAE)
Incidence of abnormal indicators of laboratory safety examinations(including blood routine, blood biochemistry, urine routine, thyroid function and coagulation function)
Geometric mean titer of SARS-CoV-2 neutralizing antibody
Geometric mean titer of SARS-CoV-2 IgG binding antibody
Seroconversion rate of the neutralizing antibody to live SARS-CoV-2 in total population
Seropositivity rate of the neutralizing antibody to live SARS-CoV-2 in total population
GMT of the neutralizing antibody to live SARS-CoV-2 in total population
Geometric mean increase (GMI) of the neutralizing antibody to live SARS-CoV-2 in total population

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Phase I/II clinical trial：The incidence of adverse events (AEs)
Phase I/II clinical trial：The incidence of adverse events (AEs) in all participants.
Phase I/II clinical trial：The incidence of serious adverse events(SAEs) in all participants.
Phase I/II clinical trial：The incidence of serious adverse events(SAEs) in all participants.
Phase I clinical trial：Changes in laboratory test indicators

Humoral immunity (IgG, neutralizing antibody)
cellular immunity

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Nucleic acid negative time
7-day/14-day nucleic acid conversion ratio
The ratio of main symptoms disappeared in 7 days/14 days
7 days/14 days chest CT imaging changes
Hospital stays

Symptoms
Body temperature
Patients status after the COVID-19 confirmed diagnosis

Netralizing antibodies against the adenovirus vector(ChAdTS)
Cellular-mediated immunity

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Occurrence of AE ≥ Grade 3
Occurrence of AEassociated with vaccination >=Grade 3
The incidence of SAE

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6 minute walk distance
strength of lower limber muscles
Quality of life
mMRC dyspnea scale
the power of gripping
Virus negative conversion rate
Antipyretic rate
Symptom relief time
Finger oxygen improvement rate
Disease progression rate
Changes in the patients' clinical status through the WHO Clinical Progression Scale (scores 0-10) at day 7
Changes in the patients' clinical status through the WHO Clinical Progression Scale (scores 0-10) at day 10
Ventilator usage rate, usage time at day 7
6min walk
PHQ-9
Respiratory symptoms
immune cells
Liver and renal function test
Incidence of adverse events within 0-28 days after the booster dose.
Incidence of serious adverse events (SAE) till the 12 months after the booster dose.
GMT of neutralizing antibodies against live SARS-CoV-2 virus on day 7 and 28 after the booster dose.
Fold increase and seroconversion of neutralizing antibodies against live SARS-CoV-2 virus on
6 minute walk distance
strength of lower limber muscles
Quality of life
mMRC dyspnea scale
Incidence of solicited AE within 14 days after the booster dose
Incidence of unsolicited AE within 28 days after the booster dose.
Incidence of serious adverse events (SAE) till the 6 months after the booster dose.
GMT of binding antibodies against SARS-CoV-2 S and N protein on day 14, day 28 and month 6 after the booster dose

Safety index-incidence of serious adverse events
Seroconversion rate,seropositivity rate and GMI of the neutralizing antibody to SARS-CoV-2
Seropositivity rate of IgG antibody

Adverse events following vaccination

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All cause mortality ascertained from data analysed to day 28
Ventilator-free days within 28 days
Negative conversion rate of SARS-CoV-2 nucleic acid in on days 14 after randomization
Average length of hospital stay
Immunogenicity index-seropositive rate of neutralizing antibodies
Immunogenicity index-seroconversion rate of neutralizing antibodies
Immunogenicity index-GMI of neutralizing antibodies (CZ02 strain)
Immunogenicity index-seropositive rate of neutralizing antibodies
Immunogenicity index-seroconversion rate of neutralizing antibodies

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Proportion of patients who progress to severe disease
Duration of fever
C reactive protein concentration in peripheral blood
Absolute value and percentage of peripheral blood lymphocytes
Time of improvement of chest imaging

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Antibody IgM, IgG levels
The neutralizing antibody S, N, total antibody
Memory B cell ratio
Imaging examination
electrocardiogram

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CD44 factor
VII factor activity
uPA / uPAR dynamic change
SARS-cov-2 neutralizing antibody GMT on day 14 after full immunization with recombinant Novel Coronavirus vaccine (CHO cells);
HI antibody GMT, positive conversion rate and serum protection rate of influenza virus typing 30 days after influenza vaccination;
Incidence of adverse reactions/events at each time point after each dose of recombinant Novel

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Neutralizing antibody as measured by pseudovirus neutralization assay
IgG antibody

Immunogenicity index-Seropositivity rate of the neutralizing antibody to live SARS-CoV-2
Immunogenicity index- GMT of the neutralizing antibody to live SARS-CoV-2
Immunogenicity index-GMI of the neutralizing antibody to live SARS-CoV-2
Immunogenicity index-Seropositive rate of the neutralizing antibody to EV71
Immunogenicity index-GMT of the neutralizing antibody to EV71

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Geometric mean increase (GMI) of the neutralizing antibody to live SARS-CoV-2
Seroconversion rate of the neutralizing antibody to live SARS-CoV-2
Seropositivity rate of the neutralizing antibody to live SARS-CoV-2
Incidence of adverse reactions 0-7 days after vaccination
Incidence of adverse reactions 0-28 days after vaccination
Incidence of solicited adverse events within 7 days after each dose.
Incidence of adverse events within 28 days after each dose.
Incidence of unsolicited adverse events within 28 days after each dose.
Incidence of serious adverse events (SAE) till the 3 months after the last vaccination.
GMT of anti-S protein of SARS-CoV-2 binding antibodies on day 28 and month 3 after the last

Safety index-Incidence of adverse reactions
Safety index-Incidence of adverse reactions
Safety index-Incidence of SAEs and AESIs

Immunogenicity index-Seropositivity rate of neutralizing antibodies(CZ02 strain)

maximum concentration (Cmax)
area under the concentration-time curve (AUC)
half-life (t1/2)
clearance (CL)
volume of distribution (V)

Netralizing antibodies against the adenovirus vector(ChAdTS)
Cellular-mediated immunity
Neutralizing Response against Variants

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Pharmacokinetics: AUC
Pharmacokinetics: Tmax
Pharmacokinetics: Cmax
Pharmacokinetics: Vd
Pharmacokinetics: CLt
IgG antibody concentration
T-cell response level
Serum neutralizing antibody positive rate
Serum neutralizing antibody positive conversion rate
Intestinal flora
IgG antibody concentration
T-cell response level
Serum neutralizing antibody positive rate
Serum neutralizing antibody positive conversion rate
Intestinal flora

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Illness Perception

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Incidence of adverse reactions within 30 minutes after the booster dose.
Incidence of adverse reactions within 14 days after the booster dose.
Incidence of adverse events within 28 days after the booster dose.
Incidence of serious adverse events (SAE) till the 6 months after the booster dose.
Fold increase and seroconversion of neutralizing antibodies against live SARS-CoV-2 virus on

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Pharmacokinetic parameters(Cmax)
Pharmacokinetic parameters(AUC0-τ)

The seropositive rate of neutralizing antibodies
GMT of neutralizing antibodies
GMFR of neutralizing antibodies
severe adverse events and adverse events of special interest
Phase I: GMT and seroconversion rate of total IgG antibody (ELISA method) against the Alpha, Beta, Delta variants SARS-CoV-2 and T4-Foldon on Day 28 after the first dose of vaccination;
Phase I: GMT and seroconversion rate of total IgG antibody (ELISA method) against the Alpha, Beta, Delta variants of SARS-CoV-2 and T4-Foldon on Day 14, Day 28, Day 90, Day

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Blood gas analysis
mMRC
Borg

Motor function

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Measure time point of outcome and Measure method of Current Secondary outcome measures (If Specified)

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The difference of PaO2/FiO2 between two groups [Time Frame: 7 days after randomization]
PaO2/FiO2 denotes ratio of arterial partial pressure of O2 and the fraction of inspired oxygen, with a higher PaO2/FiO2 means favorable outcome.
Lower Sequential Organ Failure Assessment (SOFA) score [Time Frame: 7 days
The time to fever resolution rate [Time Frame: 14 days]
Time to complete remission of fever in eligible subjects
Time to recovery of lung injury [Time Frame: 14 days]
improvement of chest radiographic evidence indirectly reflects recovery in patients infected with COVID-19

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The virological clearance rate of throat swabs, sputum, or lower respiratory tract secretions at day 3 [Time Frame: 3 days after randomization]
The virological clearance rate of throat swabs, sputum, or lower respiratory tract secretions at day 5 [Time Frame: 5 days after randomization]
Number of participants with treatment-related adverse events as assessed by CTCAE

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The disease prorogation-temperature [Time Frame: Day 0 till day 21]
Body temperature will be followed everyday during time frame.
The disease prorogation-respiratory function 1 [Time Frame: Day 0 till day 21]
Respiratory rate will be followed everyday during time frame.
The disease prorogation-respiratory function 2 [Time Frame: Day 0 till day 21]

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All cause mortality [Time Frame: up to 28 days]
baseline SpO2 during screening, PaO2/FiO2 <300mmHg or a respiratory rate ≥ 24 breaths per min without supplemental oxygen
Frequency of respiratory progression [Time Frame: up to 28 days]
Defined as SPO2≤ 94% on room air or PaO2/FiO2 <300mmHg and requirement for
Clinical status [Time Frame: days 7, 14, 21, and 28]
Clinical status, assessed by the ordinal scale at fixed time points (days 7, 14, 21, and 28).
Time to Hospital Discharge OR NEWS2 (National Early Warning Score 2) of ≤ 2 maintained for 24 hours [Time Frame: up to 28 days]

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Rate of no fever [Time Frame: two weeks]
Rate of respiratory symptom remission [Time Frame: two weeks]
Rate of lung imaging recovery [Time Frame: two weeks]
Rate of CRP,ES,Biochemical criterion (CK,ALT,Mb)recovery [Time Frame: two weeks]
Rate of no fever [Time Frame: two weeks]
Rate of respiratory symptom remission [Time Frame: two weeks]
Rate of lung imaging recovery [Time Frame: two weeks]
Rate of CRP,ES,Biochemical criterion(CK,ALT,Mb) recovery [Time Frame: two weeks]
Rate of normal tempreture [Time Frame: day 7]
Rate of patients without fever at day 7
Rate of respiratory symptom remission [Time Frame: day 7]
Rate of patients with respiratory symptom remission at day 7
Rate of lung imaging recovery [Time Frame: day 7]

time till the SARS-CoV-2 clearance [on day 3, day 6, day 9, et al; RT-PCR]

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Time of clinical remission [Time Frame: 28 days]
The definition of clinical remission:
Based on the symptoms of the disease (fever,cough,diarrhea,myalgia,dyspnea) has been relieved for 48 hours;
There is no evidence of disease progression(New dyspnea, SpO2
28-days mortality [Time Frame: on the day 28 after enrollment]
wether the patient survives
ICU length of stay [Time Frame: on the day 28 after enrollment]
days of the patients staying in the ICU
Demand for first aid measuments [Time Frame: on the day 28 after enrollment]

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Number of participants with treatment-related adverse events as assessed by CTCAE v5.0 [Time Frame: 14 days after randomization]
The critical illness rate of subjects at weeks 2 [Time Frame: 14 days after randomization]
The diagnosis of critical illness case was based on the notice on printing and

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blood pressure
heart rate [baseline, 1-week, 2-week, 3-week, 4-week;]
immunoglobin level

Others (liver and kidney function, muscle enzymes, myoglobin)[Day 1, 3, 5, 7, 10, 14, 21, 28;]

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rate of composite advers outcomes.[; Defined as SPO2≤ 93% without oxygen supplementation, PaO2/FiO2 <=300mmHg or a respiratory rate ≤30 breaths per min without supplemental oxygen (Time Frame: 14 days)]
rate of no fever (Time Frame: 14 days)
rate of no cough

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COVID-19 RNA negative conversion rate[at the end of the treatment, on the 28th day of treatment;]
Disappearance rate of main symptoms(fatigue, dyspnea)[at the end of the treatment, on the 28th day of treatment;]

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Time of virus nucleic acid test negative [Time Frame: Measured from day 0 through day 28]
Marker for 2019-nCoV
CD4+ and CD8+ T cell count [Time Frame: Measured from day 0 through day 28]
Marker of immunological function

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Arterial Oxygen Saturation[Measured once at the time of enrollment and at 6 o'clock and 14 o'clock each day after administration.;]
Critical (severe or critical) conversion rate
Inflammation index (CRP)[Once at the time of enrollment and 3, 7, and 14 days after administration (4 times in total).]
time to temperature normal[time to temperature normal; temperature measure]
breath symptoms and measure[during therapy; CT]
hospitalization days [to out hospital; count]

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Side effects in the UC-MSCs treatment group [Time Frame: From Baseline (0W) to 96 week after treatment]
Incidence of acute and chronic treatment-related adverse events in patients with novel coronavirus severe pneumonia receiving UC-MSCs infusion as assessed.
28-days survival [Time Frame: Day 28]

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Clinical remission rate [Time Frame: within 14 days from the start of medication]
Clinical remission was defined as (one of them): sustained (more than 48 hours) alleviation of illness based on symptom (fever, cough, dyspnea, myalgia, diarrhea and so on) all being absent and no evidence for progression.
Dynamic changes of oxygenation index [Time Frame: within 14 days from the start of

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#NOTREQ

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clinical cure incidence in 14 days [Time Frame: 14 days]
The clinical symptoms and signs improved or alleviated (the temperature be normal , respiratory symptoms improved significantly, imaging showed obvious absorption) and no additional or alternative treatment was needed.
the duration of virus change to negative [Time Frame: 30 days]

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The improvement proportion of pulmonary inflammation [Time Frame: Week 24]
Evaluation of pulmonary inflammation Improvement
The improvement proportion of clinical symptom [Time Frame: Week 24]
Discomfort symptoms include dyspnea, cough, exhausted, fatigue, insomnia, sweating, poor appetite, diarrhea, etc., which are common manifestations of patients

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fever[evryday; Temperature measurement]
feeble[evryday; Subject self-report]
novel coronavirus nucleic acid detection
Lung CT
Day 7, 14, 21, 28 clinical status[Day 7, 14, 21, 28; Evaluated by Grade 6 rating scale]
Time from study entry to discharge
Frequency of serious adverse drug reactions
Dynamic changes of laboratory indexes[Baseline, 7 days, 14 days, 21 days, 28 days; Blood]
Time to COVID-19 RNA convert negative
Other symptoms relief rate & time[; Weak/hacking cough]
Time & Rate of inflammatory biomarkers convert normal
Time to recovery
Frequency of antipyretic use

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Time to CoVID-19 RT-PCR negative in upper respiratory tract specimen [Time Frame: up to 28 days]
Change (reduction) in CoVID-19 viral load in upper respiratory tract specimen as assessed by area under viral load curve. [Time Frame: up to 28 days]
Time to defervescence (in those with fever at enrolment) [Time Frame: up to 28 days
The incidence of common Novel coronavirus pneumonia progressing to severe[Un the 5th, 10th day after the first dose, and discharge or turn to severe; Patients with Novel coronavirus pneumonia who meet any of the following are severe patients: respiratory distress, RR ≥30 beats / min; in a resting state, means oxygen saturation <93%; arterial blood oxygen pressure (PaO2) / oxygen concentration (FiO2))

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P/F ratio over time [Time Frame: 14 days]
PaO2/FiO2 ratio
Sequential organ failure assessment score(SOFA score) over time [Time Frame: 14 days]
SOFA, including assessment of respiratory, blood, liver, circulatory, nerve, kidney
Time to 2019-nCoV RT-PCR negativity in upper and lower respiratory tract specimen[Baseline, 7 days, 14 days, 21 days, 28 days;]
clinical status[Day 7, 14, 21, 28;]
Time from study entry to discharge
Frequency of serious adverse drug reactions

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Body temperature [Every week; Auxiliary temperature]
3CL Mpro of Coronavirus[Every week; Real - time PCR method]
Routine blood test[Every week;]
Routine urine test[Every week;]
Liver function test[Every week;]

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Inflammatory markers[Within 14 days;]
Clinical symptoms[Within 14 days;]
Liver and kidney function[Within 14 days;]

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Change in lesion proportion (%) of full lung volume from baseline to day 10 and 90 [Time Frame: Day 10, Day 90]
Evaluation of Pneumonia Improvement
Change in consolidation lesion proportion (%) of full lung volume from baseline to day 10, 28 and 90. [Time Frame: Day 10, Day 28, Day 90]
Rate of mortality within 28-days [Time Frame: At baseline, Day 1, Week 1, Week 2, Week 4, Week 8]
Marker for efficacy of treatment
Sequential organ failure assessment [Time Frame: At baseline, Day 1, Week 1, Week 2, Week 4, Week 8]

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Within 4 weeks of death from respiratory causes
Within 4 weeks of disease progression or death
Routine lymphocyte recovery[4 weeks;]
Changes in inflammatory markers of blood (il-8 et al.)[4 weeks;]
Absolute change in viral nucleic acid from baseline[4 weeks;]

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The incidence rate of complication[Long-term follow-up was 1 year; Visiting]

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The fading and obviously improving time of clinical symptoms (fever, dry cough, fatigue, etc)
The negative conversion rate of SARS-CoV-2 nucleic acid[1, 3, 7 and 14 days after infusion;]
Negative conversion time of SARS-CoV-2 nucleic acid
CRP[day1, day7 and day14;]
IL-6[day1, day7 and day14;]
TNF-α [Before and after 14 days of treatment;]
Time of improvement of clinical symptoms (fever, cough)
Adverse drug reactions[within 14 day;]

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Treatment compliance[Enrollment and two weeks after enrollment; The patient's degree of cooperation with the physician's treatment plan]
To evaluate the difference in isolation and rehabilitation of patients after discharge[7 days and 14 days after discharge; Patients were followed up for isolation and physical recovery]

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body temperature [Time Frame: 2 weeks]
inflammatory indicator

Level of IL2, IL4, IL6, IL10, TNF-alpha and IFN-gama[Everyday; ELISA]
Routine blood test[Everyday; Five-Part Differential Complete Blood Count]
lymphocyte subsets[Everyday; Flow Cytometer]

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All-cause death[28 days;]
Adverse events

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Viral nucleic acid test negative conversion rate and days from positive to negative [Time Frame: 14 days after taking medicine]
Duration of fever [Time Frame: 14 days after taking medicine]
Lung imaging improvement time [Time Frame: 14 days after taking medicine]
Mortality rate because of Corona Virus Disease 2019 [Time Frame: 3 months]

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Lung function
Arterial blood gas analysis[Days 3, 7 and 14;]

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PaO2 [Time Frame: up to 30 days]
blood oxygen saturation (SpO2) [Time Frame: up to 30 days]
clinical status rating on the 7-point ordinal scale [Time Frame: up to 30 days]
The ordinal scale is an assessment of the clinical status at the first assessment of a given study day. The scale is as follows: 1) Death; 2) Hospitalized, on invasive

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Causal mortality[4 week/8week;]
COVID-19 nucleic acid negative conversion rate[4 week/8week;]
All-cause mortality[4 week/8week;]

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Duration of fever [Time Frame: 10 days during the intervention period]
Duration of fever after recruitment
Frequencies of progression to severe illness [Time Frame: 10 days during the intervention period]
Disease is defined as severe if it meets any of the following criteria: 1 Respiratory rate

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Lung function[Base line, week 1, 2, 4 after enrollmen;]
Oxygenation index[Base line, week 1, 2, 4 after enrollment;]

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Clinical outcome [Time Frame: At baseline, 6 hour, Day 1, Day 3,Week 1, Week 2, Week 4, Month 6.]
Improvement of clinical symptoms including duration of fever, respiratory destress, pneumonia, cough, sneezing, diarrhea.
Hospital stay [Time Frame: Baseline through 6 months]

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Clinical Symptom Remission time [Time Frame: The day from admission to clinical remission,up to 2 weeks.]
Clinical Symptom Remission was defined as: The patient's body temperature was normal for more than 72 hours (without taking antipyretics or hormones) and had no conscious dyspnea or decreased dyspnea

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Detection of TNF-α levels, IL-10 levels [Time Frame: 1,3,6,12months]
Cytokines level
Detection of immune cells that secret cytokines, including CXCR3+, CD4+, CD8+, NK+ cells, and regulatory T cells (CD4 + CD25 + FOXP3 + Treg cells). [Time Frame: 1,3,6,12months]

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Lung lesion [Time Frame: 1-28 days]
Lung Lesion by CT
Immune function [Time Frame: 1-28 days]
Th1 cytokines: IL-1β, IL- 2, TNF-a, ITN-γ;
Th2 cytokines: IL - 4, IL - 6, IL - 10;

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Clinical cure rate [Time Frame: 5 months]
Definition of clinical cure: The viral load of the respiratory specimen was negative for two consecutive times (the interval between the two tests was greater than or equal to one day), the lung image improved, and the body temperature returned to normal for more than 3 days, and the clinical manifestation improved.
Changes in compound physiological index [Time Frame: 3 months]
Changes in the scores of the St. George's Hospital Respiratory Questionnaire [Time Frame: 3 months]
St. George's Hospital Respiratory Questionnaire range from 0 to 100. 0 stands for no impact on life and 100 stands for extreme impact on life

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Occurrence of adverse events [Time Frame: 0-28 days post vaccination]
Occurrence of serious adverse reaction [Time Frame: 0-6 months post vaccination]
Anti SARS-CoV-2 S IgG antibody response(ELISA) [Time Frame: 0, 14 days and 6 months post vaccination]
Neutralizing antibody response to SARS-CoV-2 [Time Frame: 0 and 6 months post
Recovery time
Virus nucleic acid negative time and rate
The rate and time of main symptoms (fever, fatigue and cough) disappear
Pulmonary imaging changes
Arterial oxygen partial pressure and oxygenation index were measured 2 days after
Incidence of abnormal indicators of liver and kidney function, blood routine and urine routine[The 4th day after each dose of vaccination in phase I;]
Rate of adverse reaction and adverse event[8-28/30 days, 0-28/30 days after inoculation of each dose of vaccine;]
Incidence of Serious Adverse Events (SAE)[Within 12 months from first vaccination to
pulmonary function
mMRC dyspnea scale
strength of lower limber muscles
ICF-COVID simple evaluation form
Quality of life[: SF-12]

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Safety indexes of adverse reactions [Time Frame: 7 days after each dose of vaccination]
Incidence of adverse reactions 7 days after each dose of vaccination
Safety indexes of Incidence of abnormal laboratory index [Time Frame: On the 3th day after each dose of vaccination in phase I]

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Covid-19 symptoms[; Visual Analogue Scale]
TCM Constitution[; TCM Constitution Judgment Scale]
Total IgG
Total IgA
IFN-α

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The proportion of responder at week 8 [Time Frame: baseline, week 8]
The responder is defined as the participants with ≥ 50% average VAS decrease of chest tightness and fatigue compared with baseline. The VAS ranges from 0 to 100.
The change in the average VAS score of main symptoms from baseline [Time Frame: baseline, week 4, week 8]

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Quality of Life[; WHOQOL-BREF]
6MWD
Lung function
Chest CT examination
Reversion rate of viral nucleic acid
Percentage of patients need for non-invasive ventilation or HFNC
Percentage of patients need for invasive ventilation
Negative conversion time of SARS-COV-2 nucleic acid test
Time for relieving fever
Time for improvement of respiratory symptoms [; Cough was quantified by Cough

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Incidence of abnormal indicators of liver and kidney function blood routine and urine routine [The 4th day after each dose of vaccination in phase I;]
Incidence of adverse reactions/events[8-28/30 days, 0-28/30 days after inoculation of each dose of vaccine;]
Incidence of Serious Adverse Events (SAE)[Within 12 months from first vaccination to

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Safety index-incidence rate of adverse reactions [Time Frame: within 7 days after each dose vaccination]
Incidence rate of adverse reactions within 7 days after each dose vaccination
Safety index-incidence rate of serious adverse events in phase I [Time Frame: From the beginning of the vaccination to 6 months after the booster immunization]
IL-6 (ng/ml) [Time Frame: baseline (at admission), day 3,7 and14 after admission or until the date of discharge or date of death from any cause]
evaluate inflammatory response, blood sample collected at 6:00am
IL-10 (ng/ml) [Time Frame: baseline (at admission), day 3,7 and14 after admission or until the date of discharge or date of death from any cause]

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Serious adverse events [Time Frame: 12 months after the second vaccination]
Occurence of Serious adverse events after vaccination
Level of Neutralizing antibodies against SARS-CoV-2 Phase Ia (Day 0, 14 schedule) [Time Frame: 7, 14 and 28 days after the second vaccination]
Level of neutralizing antibodies against SARS-CoV-2 in serum for the Phase Ia

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Primary pharmacokinetic variables [Time Frame: 12 Weeks]
Area under the curve from the time of dosing to the last measurable concentration time t (AUC0-last)
Primary pharmacokinetic variables [Time Frame: 12 Weeks]
Maximum concentration (Cmax)

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Immunogenic end point [Time Frame: Within 6 months after the last dose of vaccination]
All subjects The levels of IL-2, IL-4, IL-5, IL-6 and IFN-γ levels before each vaccination, day 14 after the first vaccination, day 7 after the second vaccination, and 1 and 6 months after the entire vaccination. All subjects before each dose of

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Neutralizing antibody GMT, positive rate [Time Frame: 14 days after inoculation]
GMT and positive rate of neutralizing antibody 14 days after the first dose of pre-vaccination people
Neutralizing antibody GMT, positive conversion rate/positive rate [Time Frame: 14 days after inoculation]

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Serious adverse events [Time Frame: 12 months after the second vaccination]
Occurence of Serious adverse events after vaccination
Seroconversion rate of Neutralizing antibodies against SARS-CoV-2 Phase Ib [Time Frame: 28 days after the second vaccination]
Seroconversion rate of neutralizing antibodies against SARS-CoV-2 in serum for

Serum Concentration of BRII-196 [Time Frame: up to 24 weeks]

Serum Concentration of BRII-198 [Time Frame: up to 24 weeks]

AUC0-t [Time Frame: 12 weeks]
Area under the curve from the time of dosing to the last measurable concentration time t (AUC0-t)
AUC0-∞ [Time Frame: 12 weeks]
Area Under the Concentration Time Curve (AUC) From Time Zero to Infinity (AUC 0-

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The proportion of subjects experiencing serious adverse events (SAEs), occurring up to Day 21 after prime vaccination and Day 28 after boost vaccination, up to Month 3, 6 and 12. [Time Frame: Day 21 after prime vaccination, Day 28 after boost vaccination, and Month 3, 6 and 12]
The proportion of subjects experiencing AE associated with BNT162b1, occurring up

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Occurrence of adverse events (AE) within 7 days after each dose of the recombinant SARS-CoV-2 vaccine (Sf9 cell) or placebo. [Time Frame: 7 days after each dose.]
Occurrence of solicited AE in the subjects within 7 days after each dose of recombinant SARS-CoV-2 vaccine (Sf9 cell) or placebo.
Occurrence of AE up to Day 28 after prime and boost vaccination. [Time Frame: Day

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Humoral immune response[; antibody titer]

Safety index-incidence of serious adverse events [Time Frame: From the beginning of the vaccination to 12 months after the second dose vaccination]
SAE will be collected throughout the clinical trial.
Immunogenicity index-seropositive rates of neutralizing antibody [Time Frame: The 28th day after each dose vaccination and the 12 month after the second dose

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Safety indexes of adverse events [Time Frame: Day 0-7,0-14,0-28 post each vaccination]
Occurrence of adverse reactions post-vaccination
Safety indexes of Hematological examination measures (Hemoglobin, WBC) [Time Frame: pre-vaccination, day 4 post each vaccination]
PK Measures:AUC0-tau [Time Frame: Day 0 to Day 85]
Area under the plasma drug concentration-time curve from time 0 to one dosing interval (AUC0-tau)
PK Measures：Cmax [Time Frame: Day 0 to Day 85]
Maximum plasma drug concentration after administration (Cmax)

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Serious Adverse Events(SAE)[Within 12 months from first vaccination to completion of full vaccination schedul;]
Abnormal indicators of blood routine, blood biochemistry, urine routine and coagulation function[Day 3 after each dose of vaccination;]
The seropositive rates and level of SARS-CoV-2 neutralizing antibody and IgG

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(VO2max) [Time Frame: The change from baseline in maximum oxygen consumption (VO2max) at maximum exercise load at Week 4 of treatment.]
The change from baseline in maximum oxygen consumption (VO2max) at maximum exercise load at Week 4 of treatment.
(VO2max) [Time Frame: The change from baseline in maximum oxygen consumption

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Incidence of adverse reactions/events[0-28 days after each dose of vaccination;]
Serious Adverse Events (SAE)[Within 12 months from first vaccination to completion of full vaccination schedule;]
Level of SARS-CoV-2 IgG antibody (tested by ELISA)[Day 28 post full vaccination;]
The seropositive rates and level of SARS-CoV-2 neutralizing antibody and IgG
Immunogenicity index-Seroconversion rate of the neutralizing antibody [Time Frame: Day 14 after the full course vaccination]
Neutralizing antibody assay will be performed using the micro-neutralization method. Seroconversion will be defined as a change from seronegative(<1:8) to seropositive(≥1:8), or ≥4 fold increase from baseline

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SARS-CoV-2 serum neutralizing titers - SCR [Time Frame: 1 Week, 6 and 12 Months after Dose 2]
Compared with baseline before Vaccination 1, SCR of SARS-CoV-2 serum neutralizing titers at 1 week, 6 and 12 months after dose 2.
SARS-CoV-2 serum neutralizing titers - GMT [Time Frame: 1 Week, 6 and 12 Months
The proportion of adverse events (AE) within 7 days after each dose of the recombinant SARS-CoV-2 vaccine (CHO cell) or placebo. [Time Frame: 7 days after each dose]
The proportion of abnormal markers of hematology, blood chemistry and urine analysis within 3 days after each dose of the recombinant SARS-CoV-2 vaccine (CHO
The incidence of adverse events (AE) [Time Frame: 0 to 28 days after vaccination in each study group]
The incidence of adverse events (AE)
The incidence of severe adverse events (SAE) [Time Frame: 0 to 28 days after vaccination in each study group]

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Time of disappearance of fever symptoms [Time Frame: From date of randomization until the date of discharge, assessed up to 6 months]
Time of disappearance of fever in degrees centigrade
The rate of conversion from COVID-19 positive to COVID-19 negative [Time Frame: From date of randomization until the date of discharge, assessed up to 6 months]

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Proportion of subjects with infusion-related reactions [Time Frame: up to 24 weeks]
Proportion of subjects with hypersensitivity reactions [Time Frame: up to 24 weeks]
Mean of change from pre-dose baseline in ECG readings [Time Frame: up to 24 weeks]
ECG readings include PR interval, RR interval, QRS interval, etc.

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The incidence of adverse events (AE) [Time Frame: from day 0 to day 30 after last dose]
The incidence of adverse events (AE)
The incidence of grade 3 adverse events (AE) [Time Frame: from day 0 to day 30 after last dose]

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Duration of COVID-19 related symptoms through Day 29 among subjects with mild-moderate COVID-19 [Time Frame: up to Day 29]
Proportion of subjects who have mild-moderate COVID-19 and develop severe COVID-19 after randomization [Time Frame: up to 26 weeks]
Assessment of PK parameters (Group A and B): maximum serum concentration

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Safety index-incidence of adverse reactions within 56 days after the first dose vaccination [Time Frame: Day 0-56 after the first dose vaccination]
Incidence of adverse reactions within 56 days after the first dose vaccination
Safety index-incidence of serious adverse events [Time Frame: Day 0-56 after the first dose vaccination]

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The incidence of adverse events (AE) [Time Frame: 0 to 30 days after vaccination in each study group]
The incidence of severe adverse events (SAE) [Time Frame: 12 months after prime and boost vaccination]
The Geometric mean titer (GMT) of anti-SARS-CoV-2 specific neutralizing antibody

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Incidence of adverse reactions within 28 days after vaccination. [Time Frame: Within 28 days after vaccination]
Incidence of adverse reactions within 28 days after vaccination.
Incidence of adverse events within 28 days after vaccination. [Time Frame: Within 28 days after vaccination]

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Incidence of Adverse Events/Reactions (AE/AR) [Time Frame: Within 0-7 days after each vaccination]
The occurence of Adverse Events/Reactions (AE/AR) in all groups
Incidence of Adverse Events/Reactions (AE/AR) [Time Frame: Within 0-28 days after each vaccination]
Geometric Mean Titer (GMT) of SARS-CoV-2 Specific Neutralizing Antibody (Live SARS-CoV-2 neutralization assay) On the 30th day after the full course of vaccination, the neutralizing antibody level (wild Strains) [Time Frame: 30th day after the full course of vaccination]
Phase 1
The Geometric Mean Titer (GMT) of anti-SARS-CoV-2 neutralizing antibody [Time Frame: 14 days after the second dose in schedule of Day 0,21,42]
Neutralizing antibody assay will be performed using the Microcytopathic assay
The four-fold increase rate of anti-SARS-CoV-2 neutralizing antibody [Time Frame: 14 days after the second dose in schedule of Day 0,21,42]

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Safety index-incidence of adverse reactions [Time Frame: Day 0-7 after each dose vaccination]
Incidence rate of adverse reactions within 7 days after each dose
Safety index-incidence of serious adverse events [Time Frame: From the beginning of the vaccination to 6 months after the second dose vaccination]

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the death in hospital [Time Frame: before and 48 hours after intervention]
the rate of death in hospital (%)
the composite endpoint of aggravation [Time Frame: before and 48 hours after intervention]
the rate of composite endpoint of aggravation(%) according to ungraded oxygen
Seroconversion rate of SARS-CoV-2 neutralizing antibody [Time Frame: 28 days after vaccination]
Seroconversion rate of anti SARS-CoV-2 specific neutralizing antibody (wild type variants test).
Incidence of solicited AE within 14 days after the booster dose [Time Frame: within 14 days after the booster dose]
Incidence of solicited adverse events (AE) within 14 days after the booster vaccination.

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Nucleic acid negative time
The ratio of nucleic acid to negative [Day 7/14]
Major symptoms disappeared [Day 7/14]
CT imaging changes of chest [Day 7/14]
Hospital stays

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Assess Pharmacokinetic parameters of LY-CovMab [Time Frame: up to 99 days]
Maximum observed concentration (Cmax)

Assess Pharmacokinetic parameters of LY-CovMab [Time Frame: up to 99 days]
Time to maximum observed concentration (Tmax)

Seroconversion rate of the neutralizing antibody to SARS-CoV-2 [Time Frame: Day 4, 14, 32,42 and 56 after the first dose]
Seroconversion rate of the neutralizing antibody to SARS-CoV-2 at different points after vaccination

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Incidence of serious AEs (SAEs) and adverse events of special interest (AESIs) [Time Frame: Day 0 to Month 13]
Percentage of participants with SAEs or AESI for 12month after last dose vaccination

Proportion of subjects with abnormal markers of hematology, biochemistry, urinalysis
Percentage of participants reporting adverse events (AEs) [Time Frame: From dose 1 through 28 days after the last dose]
Percentage of participants with solicited AEs (local, systemic) for 28 days following each primary vaccination (Days 0, 28) by intensity, relevance.

Immunogenicity index of seroconversion rate of the neutralizing antibody [Time Frame: Day 28 after the whole vaccination of COVID-19 vaccine]
Seroconversion rate of the neutralizing antibody to live SARS-CoV-2 at day 28 after the whole vaccination of COVID-19 vaccine

Incidence of adverse reactions/events [Time Frame: 0-28 days after each vaccination]
Serious Adverse Events (SAE) [Time Frame: within 12 months post full primary immunization course and the booster dose respectively]

Seroconversion rate of SARS-CoV-2 IgG binding antibody [Time Frame: 28 days
Incidence of Serious Adverse Events (SAE) [Time Frame: within 12 months post full vaccination]
Incidence of abnormal indicators of laboratory safety examinations(including blood routine, blood biochemistry, urine routine, thyroid function and coagulation function) [Time Frame: Day 3 after each dose of vaccination]

Seroconversion rate of the neutralizing antibody to live SARS-CoV-2 in total population [Time Frame: Day 28 after the second dose]
Seroconversion rate of the neutralizing antibody to live SARS-CoV-2 at day 28 after the second dose in total population

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Phase I/II clinical trial：The incidence of adverse events (AEs) [Time Frame: Day 0-7 days after each vaccination.]
Adverse events (AEs) in 0-7 days after each vaccination.

Phase I/II clinical trial：The incidence of adverse events (AEs) in all participants. [

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Virus negative conversion rate [Time Frame: 14-20 days]
virus negative conversion rate in second week, overall virus negative conversion rate

Antipyretic rate [Time Frame: 14-20 days]
defined as: the rate of Axillary temperature <37.5 ℃ for at least 48h

Changes in the patients' clinical status through the WHO Clinical Progression Scale (scores 0-10) at day 7 [Time Frame: Day 7]
The scale provides a measure of illness severity across a range from 0 (not infected) to 10 (dead)

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Incidence of adverse events within 0-28 days after the booster dose. [Time Frame: within 28 days after the booster dose.]
Incidence of adverse events (AE) within 0-28 days after the booster vaccination.

Incidence of serious adverse events (SAE) till the 12 months after the booster dose. [Time Frame: within 12 months after the booster dose.]

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Incidence of solicited AE within 14 days after the booster dose [Time Frame: within 14 days after the booster dose]
Incidence of solicited adverse events (AE) within 14 days after the booster vaccination.

Incidence of unsolicited AE within 28 days after the booster dose. [Time Frame:

Safety index-incidence of serious adverse events [Time Frame: 1 month after booster immunization]
Incidence rate of serious adverse events within 1 month after booster immunization of the Inactivated SARS-CoV-2 vaccine

Adverse events following vaccination [Time Frame: 0-6 months]
analyse the incidence of adverse events following immunization, both solicited and unsolicited

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All cause mortality ascertained from data analysed to day 28 [Time Frame: At 28 days from inclusion]
All cause mortality ascertained from data analysed to day 28
Ventilator-free days within 28 days [Time Frame: At 28 days from inclusion]
Ventilator-free days within 28 days
Immunogenicity index-seropositive rate of neutralizing antibodies (CZ02 strain) [Time Frame: Day 28 after booster vaccination]
The seropositive rate of neutralizing antibodies (CZ02 strain) 28 days after booster vaccination

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Antibody IgM, IgG levels [3 days, 7 days, 10 days, and 14 days after vaccination injection, and 14 days and 28 days after discharge]
The neutralizing antibody S, N, total antibody [14days, 28 days after vaccine injection, and 14 and 28 days after discharge]
Memory B cell ratio

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Secondary endpoint: [Time Frame: 14 days after vaccination]
SARS-cov-2 neutralizing antibody GMT on day 14 after full immunization with recombinant Novel Coronavirus vaccine (CHO cells);

Secondary endpoint: [Time Frame: 1 months after full vaccination]

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Immunogenicity index-Seropositivity rate of the neutralizing antibody to live SARS-CoV-2 [Time Frame: Day 28 after the second dose of COVID-19 vaccine]
Seropositivity rate of the neutralizing antibody to live SARS-CoV-2 at day 28 after the second dose of COVID-19 vaccine.

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Geometric mean increase (GMI) of the neutralizing antibody to live SARS-CoV-2 [Time Frame: 28 days after the full immunization]
Geometric mean increase (GMI) of the neutralizing antibody to live SARS-CoV-2 28 days after the full immunization of each lot of COVID-19 vaccine
Seroconversion rate of the neutralizing antibody to live SARS-CoV-2 [Time Frame:
Incidence of solicited adverse events within 7 days after each dose. [Time Frame: within 7 days after each dose]
Incidence of solicited adverse events within 7 days after each dose.

Incidence of adverse events within 28 days after each dose. [Time Frame: within 28
Safety index-Incidence of adverse reactions [Time Frame: 0-28 days after booster]
Incidence of adverse reactions occured 0-28 days after booster

Safety index-Incidence of adverse reactions [Time Frame: 0-7 days after booster dose]
Immunogenicity index-Seropositivity rate of neutralizing antibodies(CZ02 strain) [Time Frame: 14 days after the booster dose vaccination]
Seropositivity rate of neutralizing antibodies(CZ02 strain) 14 days after the booster dose vaccination
maximum concentration (Cmax) [Time Frame: 29 days after the last participant is randomized]
PK parameters to be evaluated for IBI314 including maximum concentration (Cmax) will be determined when appropriate.
area under the concentration-time curve (AUC) [Time Frame: 29 days after the last

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Pharmacokinetics: AUC [Time Frame: Day 1 to D85]
Area under the plasma concentration-time curve from the time of dosing to the last measurable concentration time t (AUC0-last)
Pharmacokinetics: Tmax [Time Frame: Day 1 to D85]
Time to maximum concentration (Tmax)
IgG antibody concentration [Time Frame: 8 weeks]
The IgG antibody concentration of S protein and RBD in SARS-CoV-2 serum.

T-cell response level [Time Frame: 8 weeks]
Test of T-cell response level by spot-forming cell(SFC) counting, using Champspot III

IgG antibody concentration [Time Frame: 8 weeks]
The IgG antibody concentration of S protein and RBD in SARS-CoV-2 serum.

T-cell response level [Time Frame: 8 weeks]
Test of T-cell response level by spot-forming cell(SFC) counting, using Champspot III

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Incidence of adverse reactions within 30 minutes after the booster dose. [Time Frame: Within 30 minutes after the booster dose]
Incidence of adverse reactions within 30 minutes after the booster dose.
Incidence of adverse reactions within 14 days after the booster dose. [Time Frame: Within 14 days after the booster dose]

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Pharmacokinetic parameters(Cmax) [Time Frame: 0-Day 5]
Estimate of the Cmax for single and multiple dose administration of FB2001.

Pharmacokinetic parameters(AUC0-τ) [Time Frame: 0-Day 5]
Estimate of AUC0-τ for single and multiple dose administration of FB2001

The seropositive rate of neutralizing antibodies [Time Frame: 3 days/14 days/28days/45days/60 days/90 days/180 days/365 days]
The seropositive rate of neutralizing antibodies after third vaccination shots

GMT of neutralizing antibodies [Time Frame: 3 days/14 days/28days/45days/60

Phase I: GMT and seroconversion rate of total IgG antibody (ELISA method) against the Alpha, Beta, Delta variants SARS-CoV-2 and T4-Foldon on Day 28 after the first dose of vaccination; [Time Frame: Day 28 after the first dose of vaccination]
GMT and seroconversion rate of total IgG antibody against the Alpha, Beta, Delta variants of SARS-CoV-2 and T4-Foldon on Day 28 after the first dose of vaccination

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A randomized, controlled open-label trial to evaluate the efficacy and safety of lopinavir-ritonavir in hospitalized patients with 2019-nCoV infection
No Statistically Apparent Difference in Antiviral Effectiveness Observed Among Ribavirin Plus Interferon-Alpha, Lopinavir/Ritonavir Plus Interferon-Alpha, and Ribavirin Plus
Comparative effectiveness and safety of ribavirin plus interferon-alpha, lopinavir/ritonavir plus interferon-alpha and ribavirin plus lopinavir/ritonavir plus interferon-alphain in patients with mild to moderate novel coronavirus pneumonia
Guideline-Based Chinese Herbal Medicine Treatment Plus Standard Care for Severe Coronavirus Disease 2019 (G-CHAMPS): Evidence From China
Chinese Herbal medicine for Severe nevel coronavirus pneumonia (COVID-19): a Randomized Controlled Trial
SARS-CoV-2 clearance in COVID-19 patients with Novaferon treatment: A randomized, open-label, parallel-group trial
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Effect of Convalescent Plasma Therapy on Time to Clinical Improvement in Patients With Severe and Life-threatening COVID-19: A Randomized Clinical Trial

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A multicenter, randomized, controlled trial for integrated chinese and western medicine in the treatment of novel coronavirus pneumonia (COVID-19) based on the 'Truncated Torsion' strategy

A Randomized, Single-Blind, Group Sequential, Active-Controlled Study to Evaluate the Clinical Efficacy and Safety of α-Lipoic Acid for Critically Ill Patients With Coronavirus Disease 2019 (COVID-19)

A multicenter, randomized controlled trial for the efficacy and safety of Alpha lipoic acid (iv) in the treatment of patients of severe novel coronavirus pneumonia (COVID-19)

Evaluation of the efficacy and safety of hydroxychloroquine in comparison with chloroquine in moderate and severe patients with COVID-19

Evaluation the Efficacy and Safety of Hydroxychloroquine Sulfate in Comparison with Phosphate Chloroquine in Severe Patients with Novel Coronavirus Pneumonia (COVID-19): a Randomized, Open-Label, Parallel, Controlled Trial

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Evaluation the Efficacy and Safety of Hydroxychloroquine Sulfate in Comparison with Phosphate Chloroquine in Mild and Commen Patients with Novel Coronavirus Pneumonia (COVID-19): a Randomized, Open-label, Parallel, Controlled Trial

Effect of qigong exercise and acupressure rehabilitation program on pulmonary function and respiratory symptoms in patients hospitalized with severe COVID-19: a randomized controlled trial

Liu-Zi-Jue Qigong and Acupressure Therapy for Pulmonary Function and Quality of Life in Patient with Severe novel coronavirus pneumonia (COVID-19): A Randomized Controlled Trial

Clinical efficacy of low-dose emetine for patients with COVID-19: a real-world study

A parallel, randomized controlled clinical trial for the efficacy and safety of Pediatric Huatanzhike granules (containing ipecacuanha tincture) in the treatment of mild and moderate novel coronavirus pneumonia (COVID-19)

Combination of Hua Shi Bai Du granule (Q-14) and standard care in the treatment of patients with coronavirus disease 2019 (COVID-19): A single-center, open-label, randomized controlled trial

Study on Prevention and Treatment of Novel Coronavirus Pneumonia (COVID-19) With Traditional Chinese Medicine

Negative-Pressure Isolation Mask for Endoscopic Examination During the Coronavirus Disease 2019 Pandemic: A Randomized Controlled Trial

A prospective randomized controlled trial for a Self-made Gastroscope isolation mask for preventing and controlling the novel coronavirus pneumonia (COVID-19) Epidemic period

六神丸联合常规治疗对 COVID-19 患者的临床疗效

A randomized parallel controlled trial for LIUSHENWAN in Treatment of Novel Coronavirus Pneumonia (COVID-19)

Chansu improves the respiratory function of severe COVID-19 patients

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Observation Of Clinical Efficacy And Safety Of Bufonis Venenum Injection In The Treatment Of Severe Novel Coronavirus Pneumonia (COVID-19)

Therapeutic effects of traditional Chinese medicine (Maxingshigan-Weijing Decoction) on COVID-19: An open-label randomized controlled trial

Study for the therapeutic effect and mechanism of traditional Chinese medicine in the treatment of novel coronavirus pneumonia (COVID-19)

A telerehabilitation programme in post-discharge COVID-19 patients (TERECO): a randomised controlled trial

A clinical study for the effect of remote monitoring exercise rehabilitation on the discharged patients with novel coronavirus pneumonia (COVID-19)

Efficacy and safety of Bufei Huoxue capsules in the management of convalescent patients with COVID-19 infection: A multicentre, double-blind, and randomised controlled trial

A randomized, double-blind, controlled trial for Bufei Huoxue Capsule in the treatment of convalescent COVID-19 patients

Safety and immunogenicity of the SARS-CoV-2 ARCoV mRNA vaccine in Chinese adults: a randomised, double-blind, placebo-controlled, phase 1 trial

A Phase Ib clinical trial to evaluate the safety, tolerance and preliminary immunogenicity of different doses of a SARS-CoV-2 mRNA vaccine in population aged 18-59 years

The effects of narrative exposure therapy on COVID-19 patients with post-traumatic stress symptoms: A randomized controlled trial

The Effects of Narrative Exposure Therapy on COVID-19 Patients with Post-Traumatic Stress Disorder: a Randomized Controlled Trial

Safety and immunogenicity of inactivated SARS-CoV-2 vaccine in high-risk occupational population: a randomized, parallel, controlled clinical trial

A randomized, controlled, multicenter study on the safety and immunogenicity of SARS-CoV-2 inactivated vaccine

A randomized, controlled, multicenter study on the safety and immunogenicity of SARS-CoV-2 inactivated vaccine

Efficacy and Safety of Lianhuaqingwen Capsules for the Prevention of Coronavirus Disease 2019: A Prospective Open-Label Controlled Trial

Real-world study on the prevention effect of Lian-Hua Qing-Wen Capsules on close contacts of new coronavirus pneumonia

Immunogenicity and safety of a recombinant fusion protein vaccine (V-01) against coronavirus disease 2019 in healthy adults: a randomized, double-blind, placebo-controlled, phase II trial

A Randomized, Double-blind, Placebo-controlled Phase II Clinical Trial to Evaluate the Immunogenicity and Safety of Recombinant SARS-CoV-2 Fusion Protein Vaccine (V-01) in Healthy Subjects

Safety and immunogenicity of a recombinant interferon-armed RBD dimer vaccine (V-01) for COVID-19 in healthy adults: a randomized, double-blind, placebo-controlled, Phase I trial

A Randomized, Double-blind, Placebo-controlled Phase I Clinical Trial to Evaluate the Safety and Immunogenicity of Recombinant SARS-CoV-2 Fusion Protein Vaccine (V-01) in Healthy Subjects

Favipiravir in the treatment of patients with SARS-CoV-2 RNA recurrent positive after discharge: A multicenter, open-label, randomized trial

The Mechanism, Clinical Outcome and Therapeutic Intervention of Corona Virus Disease 2019 Patients Whose Nucleic Acids Changed From Negative to Positive

Safety, Tolerability, Pharmacokinetics, and Immunogenicity of a Monoclonal Antibody (SCTA01) Targeting SARS-CoV-2 in Healthy Adults: a Randomized, Double-Blind, PlaceboControlled, Phase I Study

A Randomized, Double-blinded, Placebo-controlled, Single Ascending Dose, Phase I Study to Evaluate the Tolerability, Safety, Pharmacokinetics of SCTA01 in Healthy Subjects

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Safety, tolerability, pharmacokinetic characteristics, and immunogenicity of MW33: a Phase 1 clinical study of the SARS-CoV-2 RBD-targeting monoclonal antibody

A Phase I Clinical Study to Evaluate the Safety, Tolerability, Pharmacokinetic Characteristics and Immunogenicity of a Single Dose of MW33 Injection in Healthy Subjects

A Multicenter, Randomized, Double-Blind, Placebo-Controlled Phase 2 Clinical Trial of MW33 Injection to Evaluate the Efficacy and Safety in Patients With Mild or Moderate COVID-19

Safety, tolerability, and immunogenicity of an inactivated SARS-CoV-2 vaccine (CoronaVac) in healthy children and adolescents: a double-blind, randomised, controlled, phase 1/2 clinical trial

A Randomized, Double-Blinded, Placebo-Controlled, Phase I/II Clinical Trial, to Evaluate the Safety and Immunogenicity of the SARS-CoV-2 Inactivated Vaccine (Vero Cell) in Healthy Population Aged 3-17 Years

Safety and immunogenicity of a recombinant adenovirus type-5-vectored COVID-19 vaccine with a homologous prime-boost regimen in healthy participants aged 6 years and above: a randomised, double-blind, placebo-controlled, phase 2b trial

A Randomized, Double-blind, Placebo -Controlled Phase IIb Clinical Trial to Evaluate the Safety and Immunogenicity of Ad5-nCoV in Person 6 Years of Age and Older and Those Who Have Previously Been Vaccinated With Ad5-EBOV

Heterologous AD5-nCOV plus CoronaVac versus homologous CoronaVac vaccination: a randomized phase 4 trial

Safety and Immunogenicity of Sequential Immunization of Inactivated SARS-CoV-2 Vaccine and Recombinant SARS-CoV-2 Vaccine (Ad5 Vector) in Chinese Healthy Adults Aged 18-59 Years: a Randomized, Observer-blind, Parallel-controlled Clinical Trial

A Randomized, Double-Blind, Placebo-Controlled, First-in-Human Clinical Trial to Assess Safety, Tolerability, and Pharmacokinetics of LY-CovMab, a Potent Human Neutralizing Antibody Against SARSCoV-2

A Phase I, Single Ascending Dose Study to Evaluate the Safety, Tolerability, Pharmacokinetics, and Immunogenicity of LY-CovMab Injection in Chinese Healthy Subjects

Safety and immunogenicity of a third-dose homologous BBIBP-CorV boosting vaccination: interim results from a prospective open-label study

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Reactogenicity and Immunogenicity of Third Dose Vaccine Booster Following Two Doses of Inactivated Vaccines

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Deviation in Estimated Enrollment (1 if Yes, 0 if not specified in one or more of data sources)	Absoulute Deviation by Precentage	Actual Enrollment (Participants Randomized)/Original Estimated Enrollment in Registry
1	24.40%	199 (The planned enrollment of 160 patients in the trial occurred quickly, 160
1	6.50%	101 (the efficient control and suppression of the COVID-19 outbreak in 108
		42 42
1	1.10%	89 90
1	30.80%	157 120
1	4%	96

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		100
		60
		60
1	48.50%	103 (The last patient enrolled in this study was on March 27, 2020, and for the next 3 days, we were
		200
		20
		20
		360
		360
1	91.70%	10
1	51.70%	58
		120

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		200
		200
1	48.30%	31
		60
1	93.30%	31
		460
1	95%	10
1	75%	50
		200
1	51.70%	273
		180
		60

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		60
1	82.70%	26
		150
1	78%	96 (phase 1) and 224 (phase 2)
1	23.10%	784 (Phase I), 336 (Phase II)
		1456
1	69.90%	phase I: 192, phase II: 448
1	52.60%	288 (Phase I), 720 (Phase II)
		2128
		144
		144
1	52.50%	phase I: 60, phase II: 500

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		180
		1000
1	50%	500
		1000
		93
		93
1	68%	48
		150
		30
		30
1	31.20%	86 (Based on the estimated number of patients admitted to the hospital at that time, we initially

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		125
1	47.60%	237
		452
		30
		30
1	60%	56 (With the control of the epidemic, this trial was stopped early, and the number of qualifying
		140
1	78.50%	86 (Because the number of patients with COVID-19 pneumonia in all the centers decreased in late
		400
1	70%	18 (The number of patients finally included was less than expected due to the successful containment of
		60

1	124.40%	101
1	124.40%	101
		45
1	1.60%	508
		500
		744
		744
		422
		422
1	27.70%	18–59 years: 540 60 years and older: 303
		744
		422

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	750
	192
	942
	40
	40
	50 (phase I), 900 (phase II)
	50
	900
	168 (Phase I), 960 (Phase II)
	168
	960
	30

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		30
1	69.30%	123
1	29.30%	283
		400
		408
		408
1	23.80%	122
		160
1	75%	17
		68
1	56%	88
		100

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		100
1	36%	128
		200
1	61%	39
		100
		204
		204
1	6.70%	320
		300
1	16.70%	80
		96
		50

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		50
1	15.70%	59
		70
		120
		120
1	9.20%	131
		120
		120
		120
		111
		111
1	12.40%	809

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		360
		360
1	10.20%	1976
		2200
		880
		880
		180
		180
1	73.80%	55
		210
		33
		33

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1	76.90%	42
		32
		150
		72 (Phase I), 480 (Phase II)
		552
1	10.60%	430
		481
		300
		300
		42
		42
1	74.30%	103

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400

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Deviation in Center (1 if Yes, 0 if not specified in one or more of data sources)	Center	Deviation in Inclusion Criteria (1 if Yes, 0 if not specified in one or more of data sources)
	Jin Yin-Tan Hospital Wuhan Jinyintan Hospital (Wuhan Infectious Diseases Hospital)	1
	Chongqing Public Health Medical Center Chongqing Public Health Medical Center	1
	Hubei Provincial Hospital of Integrated Chinese and Western Medicine Hubei Provincial Hospital of Integrated Chinese and Western Medicine	
1	First Hospital of Changsha The First Hospital of Changsha The Second Xiangya Hospital of Cental South University	1
0	Not specified Hubei Integrated Traditional Chinese and Western Medicine Hospital Huanggang Central Hospital (Dabieshan Medical Center)	1
0	Not specified	

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	West China Hospital, Sichuan University Chengdu Public Health Clinical Medical Center Wuhan Red Cross Hospital	
0	Not specified	1
	The Fourth People's Hospital of Wuhan	
1	Union Hospital of Tongji Medical College of Huazhong University of Science and Technology; Union Hospital, Tongji Medical College, Huazhong university of Science and Technology Red Cross Hospital in Wuhan	
	Guangshan County People's Hospital People's Hospital of Guangshan County	
0	Not specified	1
	Suizhou Central Hospital Xiaogan Central Hospital Wuhan 7th Hospital Ezhou Central Hospital Jingzhou First People's	
1	Beijing YouAn Hospital, Capital Medical University	1
1	Beijing YouAn Hospital, Capital Medical University Puren Hospital Affiliated with Wuhan University of Science beijing youan houspital Wuhan PuRen Hospital Shanghai Guanghua integrated traditional Chinese	1

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1	Guangzhou No. 8 People's Hospital [Guangzhou, China], Wuhan Union Hospital, and Wuhan Hankou Hospital	1
	The First Affiliated Hospital of Guangzhou Medical University	
	Guangzhou Eighth People's Hospital	
	the Fourth People's Hospital of Nanning	1
	The fourth people's hospital of nanning, guangxi	
1	Dawu County People's Hospital Huangpi Chinese Medicine Hospital	0
	The First Affiliated Hospital of Guangzhou Medical University Wuhan University People's Hospital	
0	Not specified	1
1	Renmin Hospital of Wuhan University	1
	Renmin Hospital of Wuhan University Zhongnan Hospital of Wuhan University	
1	Affiliated Tongji Hospital of Huazhong University of Science and Technology, Wuhan Institute for	
	People's Hospital of Wuhan University Central Hospital of Chongqing Three Gorges	
	the First people's Hospital of Jingzhou	1

	Jingzhou First People's Hospital	
0	Not specified	1
	Guiqiang Wang Peking University First Hospital Ezhou Central Hospital Huoshenshan Hospital of	
1	CDC of Wuzhi Country	1
1	Wuzhi County Center for Disease Control and Prevention, Henan Province Wuzhi County Center for Disease Control and Prevention People's hospital, Wuzhi	1
	Shangqiu City Liangyuan District Center for Disease Control and Prevention	1
	the Shangqiu City Liangyuan District Center for Disease Control and Prevention	1
	Liangyuan district centers for disease control and prevention	
	Taizhou Vaccine Clinical Research Center	1
	Jiangsu Provincial Center for Disease Control and Prevention	
	Jiangsu Provincial Center for Disease Control and Prevention	1

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	Huaiyin District Center for Disease Control and Prevention, Huaian City	
	Huaiyin District Center for Disease Control and Prevention, Huaian City	
	Jiangsu Provincial Center for Disease Control and Prevention (JPCDC)	1
	Huaiyin District Center for Disease Control and Prevention, Huaian City	
	the First Affiliated Hospital of Bengbu Medical College	1
	The First Affiliated Hospital of Bengbu Medical College	
	the Fifth Medical Center of Chinese PLA General Hospital	1
	The Fifth Medical Center, General Hospital of PLA	
	Shanghai Public Health Clinical Center (SPHCC)	
	Shanghai Public Health Clinical Center	
	Guangzhou Eighth People's Hospital	1

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	Guangzhou Eighth People's Hospital	
0	Not specified	1
	Not specified	
	Shanghai Public Health Clinical Center Shanghai Public Health Clinical Center	
1	Zhongnan Hospital of Wuhan University, Leishenshan (Thunder God Mountain) Hospital Zhongnan Hospital of Wuhan University	1
0	Not specified	1
	Hubei province hospital of integrated Chinese & Western Medicine Yichang first people's Hospital	
	The Second Affiliated Hospital of Wenzhou Medical University	1
	The Second Affiliated Hospital of Wenzhou Medical University	

0	Not specified	1
0	Not Specified	0
	General Hospital of Central Theater Command Maternal and Child Hospital of Hubei Province	
0	Not specified	1
	Hubei Provincial Center for Disease Control and Prevention	
	Jiangsu Provincial Center for Disease Control and Prevention (CDC) in Suining County Suining County Center for Disease Control and Prevention	
	Hebei Provincial Center for Disease Control and Prevention (CDC) in Renqiu (Hebei, China). Renqiu City Center for Disease Control and Prevention	
	Jiangsu Provincial Center for Disease Control and Prevention (CDC) Hebei Provincial CDC Suining County Center for Disease Control and Prevention Renqiu City Center for Disease Control and Prevention	

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1	the CDCs of Mile and Gejiu County of Yunnan Province	1
0	Not specified	1
	West China Second University Hospital, Sichuan University West China women's and children's Hospital	
0	Not specified	0
	Huashan Hospital affiliated to Fudan University	
1	The phase 1 trial was done at two hospitals in China: The Second Affiliated Hospital of Chongqing Medical The Second Affiliated Hospital of Chongqing Medical University Hunan Provincial Center for Disease Control and Prevention	1
0	Not Specified	1
	Jiangsu Provincial Center for Diseases Control and Prevention Jiangsu Provincial Center for Diseases Control and Prevention	
	The First Affiliated Hospital, Zhejiang University School of Medicine	1

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	The First Affiliated Hospital, Zhejiang University School of Medicine	
1	the Hongshan and Wuchang districts of Wuhan City, Hubei Province	1
0	Not Specified	1
	Hubei Provincial Hospital of TCM	
	Dongxihu FangCang hospital, Wuhan Keting, Hubei Province	1
	Wuhan Keting Square Cabin Hospital	
1	Leishenshan Hospital of Wuhan Tongji Hospital Wuhan Mental Health Center Huangshi Hospital of traditional Chinese Medicine Tongji Hospital of Huazhong University of science and	1
1	Wuhan JinYinTan Hospital	1
	Wuhan Jinyintan Hospital Renmin's Hospital of Wuhan University Tongji Hospital Affiliated to	
1	Peking University Third Hospital Peking University People's Hospital Tongji Hospital of Tongji Medical College, Huazhong University of Science and Technology	1

	Tongji Hospital of Tongji Medical College, Huazhong University of Science and Technology	
	Huangshi Hospital of Traditional Chinese Medicine	1
	Huangshi city hospital of TCM	
1	Wuhan Fangcang Shelter Hospital	
	First Affiliated Hospital of Anhui Medical University	
	Wuhan mobile cabin hospital	
	Binhu hospital of the first	
	the Wuhan Jinyintan Hospital	1
	Wuhan Jinyintan Hospital	
	the Endoscopy Center of West China Hospital, Sichuan University	1
	West China Hospital, Sichuan University	
	武汉市雷神山医院	1
	Leishenshan hospital	
	the First People's Hospital of Jiangxia District, Wuhan	

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	The first people's Hospital of Jiangxia District, Wuhan	
1	First Affiliated Hospital of Wenzhou Medical University	1
	The First Affiliated Hospital of Wenzhou Medical University Wenzhou city center hospital medical group	
	Jiangsu Province Hospital/Nanjing Medical University First Affiliated Hospital	1
	Jiangsu Province Hospital Hubei Province Hospital Intergrated Chinese & Western Medicine	
0	Not Specified	
	The First Affiliated Hospital of Guangzhou Medical University Xiangzhou District People's	
	Shulan (Hangzhou) hospital in Hangzhou	1
	Shulan (Hangzhou) Hospital	
0	Not Specified	
	Xiangyang Central Hospital Xiangyang Hospital of Traditional Chinese Medicine	
0	Not Specified	1

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	Outpatient Department of Shanxi Aviation Industry Group Co. LTD Outpatient Department of Shanxi Aviation Industry Group Co. LTD	
0	Not Specified	
	The Second Hospital of Hebei Medical University	
0	Not Specified	1
	Gaozhou Municipal Center for Disease Control and Prevention	
	the Guangdong Provincial Center for Disease Control and Prevention Gaozhou Municipal Center for Disease Control and Prevention	1
1	Wuhan Pulmonary Hospital Ezhou Central Hospital The Third People's Hospital of Shenzhen The Second People's Hospital of Fuyang Ezhou Hospital of Traditional Chinese Medicine	1
0	Not Specified	
	Beijing SHIJITAN Hospital	

	Shanghai Public Health Clinical Center	1
	Shanghai Public Health Clinical Center	
	Shanghai Public Health Clinical Center	
	Zanhuang CDC	
	Zanhuang county Center for Disease Control and Prevention	
0	Not Specified	
	Taixing City center for Disease Control and Prevention	
	Lianshui Center for Disease Control and Prevention	
	Jiangsu Provincial Center for Disease Control and Prevention	
	the Second Hospital of Anhui Medical University of China	
	The Second Hospital of Anhui Medical University	
1	Huashan Hospital, National Medical Center for Infectious Diseases [NMCID], Shanghai, China	1

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Huashan Hospital affiliated to
Fudan University
Beijing Ditan Hospital Capital
Medical University

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Inclusion Criteria

Male and nonpregnant female patients 18 years of age or older were eligible if they had a diagnostic specimen that was positive on RT-PCR, had pneumonia confirmed by chest imaging, and had an oxygen saturation (Sao2) of 94% or less while they were breathing

1. Adult aged >=18years old;
2. Patients infected with new coronavirus confirmed by PCR
3. In the state of no oxygen at rest, the patient's SPO2<=94% or the oxygenation index is less than 300mmHg.

(1) 18–65 years of age; (2) diagnosed as mild to moderate COVID-19; and (3) willing to sign informed consent

(1) 18â€“65 years of age;
(2) Diagnosed as mild to moderate COVID-19;
(3) Be willing to sign informed consent;
(4) The clinical assessment and condition of the subject does not influence the completion of the study

adult patients (≥18 years),
positive test result for SARS-CoV-2 on a polymerase-chain-reaction (PCR) assay,
respiratory rate (RR) ≥30/min or SaO2 ≤ 93% or a PaO2/FiO2 ratio ≤ 300 mmHg (7),
and able to provide informed consent

1) Aged >= 18 years;
2) Meet the diagnostic criteria of severe pneumonia patients with new coronavirus;
3) The informed consent form.

Hospitalized COVID-19 patients with confirmed SARS-CoV-2 detection, clinically classified as moderate or severe, at an age over 18 years, and without comorbidity of severe heart, lung, or brain diseases, were eligible for enrolling into this study.

(1) aged 18 to 70 years;
(2) Light and heavy patients with new coronavirus infection confirmed by etiology.

(I) diagnosed with COVID-19;
(II) one or more symptoms among fever, cough, and fatigue at enrollment;
(III) the time from onset to diagnosis time was not more than 48 h; and
(IV) ≥18 years of age

(1) Comply with the diagnostic criteria of "Pneumonitis Diagnosis and Treatment Scheme for New Coronavirus Infection (Trial Version 5)";
(2) The classification was judged as common and severe type;
(3) Having one or more symptoms of fever, cough and fatigue when entering the group;
(4) The diagnosis time does not exceed 48 hours;

Eligible patients were males and non-pregnant females aged 18 years or older, diagnosed with moderate-to-severe COVID-19 pneumonia according to the Diagnosis and Treatment Protocol for Novel Coronavirus Pneumonia released by the National Health Commission of China.

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1. Age over 18 years old;
2. Real-time fluorescent RT-PCR for respiratory or blood specimens to detect novel coronavirus nucleic acid positive;
3. The sequence of virus genes in respiratory or blood samples was highly homologous with the known novel coronavirus;

Inclusion criteria were adults (aged >18 yr), with clinically-confirmed COVID-19 pneumonia and hypoxaemia (defined as the ratio of arterial oxygen tension [Pao 2] to inspiratory oxygen fraction [F io 2] <300 mm Hg), and requiring intubation in the ICU.

1. Eligible patients were adults ((aged 18-80 years);
2. Were identified as laboratory-confirmed 2019-nCoV infection;
3. Those who need intubation in ICU, and with no severe hypoxemia.

(1) signed informed consent;
(2) aged at least 18 years;
(3) COVID-19 diagnosis based on polymerase chain reaction (PCR) testing;
(4) positive PCR result within 72 hours prior to randomization;

1. The patient signed an informed consent form to participate in the study of convalescent plasma therapy;
2.The age of the patient is 18 or more than 18 years old;
3. COVID-19 patients diagnosed by PCR;
4. Nucleic acid positive within 72 hours before blood transfusion;

1) age 18 and over, regardless of gender;
2) respiratory or blood samples that were tested positive for SARS-CoV-2 nucleic acid by RT-PCR, or respiratory or blood samples that were tested highly homologous with the known SARS-CoV-2 by viral gene sequencing;

(1) aged >=18 years old;
(2) real-time fluorescence rt-pcr of respiratory or blood samples presents the positive nucleic acid of COVID-19, or the viral gene sequencing of respiratory or blood samples was highly homologous with the known COVID-19.
(3) patients diagnosed with novel coronavirus meet the diagnostic criteria of the latest

Inclusion criteria were age 18 years or older, ongoing SARS-CoV-2 infection confirmed in upper or lower respiratory tract specimens with real time reverse transcriptase polymerase chain reaction (RT-PCR), willingness to participate, and consent not to be enrolled in other clinical trials during the study period

1. Aged 18 years old and above;
2. Meet the novel coronavirus pneumonia (COVID-19) diagnostic criteria. The upper and lower respiratory tract RT-PCR confirmed that 2019-nCoV nucleic acid positive, chest CT imaging examination could be used in conjunction;
3. SaO2/SpO2=94% under indoor air, or PaO2/FiO2<300mgHg (for severe type patients);

We initially enrolled patients with COVID-19 (age 18-95 years) according to the guidance of National Health Commission of China (Table 1). If no improvement signs were observed under the standard treatments, the patient would be suggested to join this polit study.
(1) age range 18–95
(2) confirmed as infected with SARS-CoV-2 by qRT-PCR
(3) confirmed as having common, severe, or critically severe COVID-19 according to the guidelines of the National Health Commission of China

1. Male or female, 18-95 years old;
2. 2019 new coronavirus nucleic acid test positive;
3. Diagnosed as moderate to severe cases of new coronavirus pneumonia in 2019, which meets the following clinical manifestations:

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Patients 18 years or older who had positive test results with reverse-transcription polymerase chain reaction (RT-PCR) assay for severe acute respiratory syndrome coronavirus 2 (SARS-CoV-2) in oropharyngeal samples were eligible if they had pneumonia as confirmed by chest imaging and a PBL cell count of 800 per uL (to convert to ×109/l

1. Patients aged 15-80 years,diagnosed with mild, common, and severe novel coronavirus pneumonia in accordance with the diagnostic criteria for the diagnosis and treatment of novel coronavirus pneumonia issued by the National Health Commission (trial version 5);
2. White blood cell Count < 15.0 x 10^9/L and Lymphocyte Count < 0.8 x10^9/L.

had a diagnosis of moderate COVID-19 (combined with epidemic history, with fever, respiratory symptoms, nucleic acid test positive, radiological findings of pneumonia, but did not match severe or critical diagnosis); agreed to collect oropharyngeal swabs and venous blood

Patients aged >=18 years with novel coronavirus pneumonia who agreed to participate in this trial and signed the informed consent.

Not specified

Suspected cases of neo-coronary pneumonia (outside Hubei province), clinically diagnosed cases (in Hubei province), and light / general confirmed cases, severe/critical confirmed cases, aged 18-70 years, each center enrolls cases in a competitive manner.
The diagnostic criteria for the new coronavirus pneumonia conform to the diagnostic criteria issued by the National Health and Medical Commission for the diagnosis and treatment of

(1) Men or women, 18–70 years old; (2) meet the diagnostic criteria of a confirmed case according to national diagnostic and treatment program; (3) within 10 days of onset; (4) subjects who can take oral medication or receive oral medication; (5) non-pregnant women, and effective contraception within 7 days of the last drug use to ensure that no pregnancy is

(1) aged 18 -70 years with a diagnosis of COVID-19 conforming to the Chinese Guidelines;
(2) hospitalized for prolonged post-symptomatic viral shedding;
(3) able to orally take medication;
(4) non-pregnant women;

1. aged 18-70 years male or female;
2. 2019 new coronavirus nucleic acid test positive;
3. meet the national diagnosis criteria;
4. subjects onset within 8 days (severe cases can be delayed to 14 days);

The inclusion criteria those who were aged between 18-75 years (including threshold), regardless of gender; were under isolation observation while diagnosed with a mild or common type of COVID-19 and could cooperate to complete a corresponding psychological intervention; had mild to moderate depressive or anxiety symptoms as defined by the 17-

1. Age between 18 and 75 years old (including threshold), regardless of gender;
2. The patient under isolation observation is diagnosed with mild or common type of 2019-nCoV acute respiratory disease and can cooperate to complete corresponding psychological intervention;

1 Informed written consent from the patients themselves or their close relatives;
2 Age range from 18 to 80 years;
3 Diagnostic criteria for novel coronavirus pneumonia in the diagnosis and treatment guidelines for severe COVID-19 patient (version sixth) developed by the National Health

1. The patient signs the written informed consent;
2. Aged 18-80 years;
3. the novel coronavirus novel coronavirus infection diagnostic criteria for pneumonia, namely, the new coronavirus pneumonia diagnosis and treatment plan (trial version sixth). It conforms to any of the following: 1) in case of shortness of breath, RR >= 30 times / min; 2)

①Laboratory-confirmed cases according to Chinese guidelines of COVID-19;
②Male or female more than 18 years old;
③Increased interleukin-6

Clinically diagnosed with Corona Virus Disease 2019
Increased interleukin-6
Sign the informed consent
Subjects who can take medicine orally
Agree to collect clinical samples

Age range: healthy people aged 18-59 years old
General good health as established by medical history and physical examination;
Since December 2019, the participant has not gone to Hubei province, overseas, or to a village/community where there had been COVID-19 cases, has not contacted with

Age range: healthy people aged ≥18 years old
General good health as established by medical history and physical examination;
Since December 2019, the participant has not gone to Hubei province, overseas, or to a village/community where there had COVID-19 cases, have not contacted with confirmed or

1. Healthy subjects aged 6 years and above;
2. Request medical history and physical examination, and the researchers judged health status of subjects;
3. Since December 2019, subjects have not been to Hubei, overseas and

Eligible participants were healthy people aged 18–80 years, who were negative for serum-specific IgM/IgG antibodies against SARS-CoV-2, as measured by a commercial kit (Innovita, China) at the time of screening.
The enrolled participants were healthy individuals aged 3–17 years, who were negative for serum-specific IgM or IgG antibodies against SARS-CoV-2 N and S proteins (tested and verified with a commercial kit from Innovita, China) at the time of screening and vaccine inoculation on the same day

1. Healthy subjects aged 3 years and above;
2. By asking for a medical history and a physical examination, the researchers determined that the subjects were in good health;
3. From December 2019 to now, the subject has not been to hubei province, outside the country or in a village/community where there has been an outbreak. The subject has not

1. Male or female subjects of ≥18 years old and ≤55 years old with body mass index (BMI) ≥18 and ≤30 at the Screening Visit.
2. Individuals who are in good health condition at the time of entry into the trial as determined by medical history, physical examination (including vital signs, ECG) and

For adult group (age ≥18 and ≤55)

Male or female subjects of ≥18 years old and ≤55 years old with body mass index (BMI) ≥18 and ≤30 at the Screening Visit.
Individuals who are in good health condition at the time of entry into the trial as determined

Eligible participants were healthy adults aged 18 to 59 years who were seronegative for anti-SARS-CoV-2 immunoglobulin M and immunoglobulin G (IgG) and negative for SARS-CoV-2 nucleic acid as confirmed by pharyngeal swab reverse transcription polymerase chain reaction. To be included, participants were required to have an axillary temperature of

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1. Healthy permanent residents aged 18 years and above;
2. Subjects agree to sign the informed consent forms voluntarily;
3. Subjects are able to comply with the requirements of the clinical trial protocol;
4. Armpit temperature <=37.0 degree C;
5. Female subjects of childbearing age were not pregnant at the time of enrollment, were

1. Healthy permanent residents aged 18 years and above;
2. Subjects agree to sign the informed consent forms voluntarily;
3. Subjects are able to comply with the requirements of the clinical trial protocol;
4. Armpit temperature <= 37.0 degrees C;
5. Female subjects of childbearing age were not pregnant at the time of enrollment, were

Eligible participants were healthy individuals aged 18 through 59 years, and 60 years or more who were seronegative for SARS-CoV-2 IgM and IgG and negative for SARS-CoV-2 nucleic acid as confirmed by pharyngeal swab reverse transcription polymerase chain reaction (RT-PCR).

1. Healthy permanent residents aged 18 years and above;
2. Subjects agree to sign the informed consent forms voluntarily;
3. Subjects are able to comply with the requirements of the clinical trial protocol;
4. Armpit temperature <= 37.0 degrees C;

(1) positive for the COVID-19 by the real-time fluorescent reverse transcription polymerase chain reaction (RT-PCR) test for the detection of nucleic acid in respiratory specimens with throat swabs and sputum specimens,

(2) patients with COVID-19 who had mild symptoms in line with the diagnostic criteria of the

1. Patients with COVID-19 who had mild symptoms in line with the diagnostic criteria of the Chinese Management Guidelines for COVID-19 (version 6.0)
2. Good communication and understanding of Chinese
3. Aged 20-69 years

Participants were recruited from "suspected COVID-19 patients" in the Fifth Medical Center of Chinese PLA General Hospital who were newly tested positive for SARS-CoV-2.

Confirmed COVID-19 infection case in term of laboratory evidence;
80 years ≥ age ≥ 14years;
Within 72 hours after the onset of abnormalities shown by Chest radiology or several symptoms (fever and cough).

All the participants had laboratory-confirmed SARS-CoV-2 infection and were willing to participate the study, as evidenced by signing an informed consent.

The participants were diagnosed as COVID-19 pneumonia, according to the notice on printing and distributing the diagnosis and treatment plan of pneumonia with new coronavirus infection (trial version 4 or update version) made by National Health Commission of the People's Republic of China.

Written the informed consent

1) age between 18 and 80 years;
2) SARS-CoV-2 infection confirmed by real-time PCR (RT-PCR) from pharyngeal swab;
3) mild clinical status, defined as having mild clinical symptoms but no signs of pneumonia on imaging or moderate clinical status, defined as having fever, respiratory symptoms and

In sputum, throat swab, lower respiratory tract secretion, blood and other samples, the nucleic acid of the novel coronavirus was positive, or the sequencing of the virus gene was highly homologous with the known novel coronavirus
Age is between 18-80 years old, the weight is more than 30kg, and there is no limit for men and women

Eligible patients were men and non-pregnant women with COVID-19 who were aged at least 18 years and were RT-PCR positive for SARS-CoV-2, had pneumonia confirmed by chest imaging, had oxygen saturation of 94% or lower on room air or a ratio of arterial oxygen partial pressure to fractional inspired oxygen of 300 mm Hg or less, and were within

Age ≥18 years at time of signing Informed Consent Form
Laboratory (RT-PCR) confirmed COVID-19.
Lung involvement confirmed with chest imaging
Hospitalized with a SaO2/SPO2≤94% on room air or Pa02/Fi02 ratio <300mgHg
≤12 days since illness onset

年龄≥18岁，根据“诊疗方案”确诊COVID-19并签署知情同意书

The participants were diagnosed as COVID-19 pneumonia, according to the notice on printing and distributing the diagnosis and treatment plan of pneumonia with new coronavirus infection (trial version 4 or update version) made by National Health Commission of the People's Republic of China;

Participants aged over 18;
(1) age ≥ 18 and < 80 years;
(2) RT-PCR positive for SARS-CoV-2;
(3) pneumonia confirmed by chest imaging and admission to the ICU;
(3) PaO2/FiO2(P/F) < 300 mmHg.
≥ 18 years old;
Diagnosed as serious or critical SARI (according to the 4th version of Diagnosis and Clinical management of 2019-nCoV infected pneumonia);
Being treated in the ICU.

Patients with COVID-19 pneumonia aged 18 years or older, admitted to the general wards for less than 72 h, and able to sign informed consent were eligible for this trial.

age more than 18 years old
accord with the clinical diagnosis and/ or etiological diagnosis diagnostic criteria of Novel coronavirus pneumonia (COVID-19).
admitted in the general wards
be able to sign informed consent

(1) age ≥ 18 years but ≤ 80 years;
(2) confirmed or clinically suspected mild or moderate coronavirus pneumonia (COVID-19), based on China’s Novel Coronavirus Pneumonia Diagnosis and Treatment Plan (Provisional 5th Edition); and
Aged between 18 and 80 years (including 18 and 80years, male or female).

One of them:
Laboratory (RT-PCR) and clinical symptoms confirmed case of novel coronavirus pneumonia (COVID-19)

(1) severe COVID-19 diagnosed after onset of disease;
(2) chest computed tomography (CT) imaging confirmed pneumonia combined with lung damage.

Not Specified

Male or female, aged at 18 years (including) -75 years old
Hospitalized
Laboratory confirmation of SARS-CoV-2 infection by reverse-transcription polymerase chain reaction (RT-PCR) from any diagnostic sampling source

Eligible participants were healthy adults aged 18 years or older, who were HIV-negative and previous SARS-CoV-2 infection-free, confirmed by commercial human immunodeficiency virus antibody detection kit (InTec products, Xiamen, China) and SARS-CoV-2 rapid test kit (Jinwofu, Beijing, China) using fingertip blood at screening. To be included, participants

Aged between 18 and 60 years.
Able to understand the content of informed consent and willing to sign the informed consent
Able and willing to complete all the secluded study process during the whole 6 months study follow-up period.
Negative in HIV diagnostic test

Participants were eligible if they were healthy and aged 18–59 years.

Healthy adults aged 18-59 years;
Proven legal identity;
Participants should be capable of understanding the informed consent form, and such form should be signed prior to enrolment ;

Eligible participants were healthy adults aged 60 years or older.

Healthy adults aged ≥60 years;
Be able to understand and sign the informed consent voluntarily;
Provide legal identification;

18-59
(1) Healthy subjects aged 18-59;
(2) Able to understand and sign the Informed Consent Form voluntarily;
(3) Provide legal proof of identity.

Healthy adults aged 18-59 years;
Proven legal identity;
Participants should be capable of understanding the informed consent form, and such form should be signed prior to enrolment ;

Healthy adults aged ≥60 years;
Be able to understand and sign the informed consent voluntarily;
Provide legal identification;

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Healthy volunteers aged 18 to 59 years were eligible for enrollment with written informed consent.

Healthy volunteers 18–59 years of age were eligible for enrollment after providing informed consent.

Phase Ia:

Healthy adults aged 18 to 59 years (including boundary values), both men and women.
Proven legal identity.
Participants should understand the contents of the informed consent form, the vaccine in

Not specified

Male and female subjects aged 18 to 45 years, inclusive;
The body weight no less than 50 kg for male subjects and no less than 45 kg for female subjects. Body mass index (BMI) = weight (kg)/square of height (m2), ranging from 18-28 kg/m2 (including the critical value);
Normal or abnormal but clinically insignificant physical examination, vital signs, laboratory

Phase I
1) According to the observation age of this clinical trial: 18 59 years old (including) adults;
2) The subjects voluntarily agreed to participate in the study, signed the informed consent form, and provided valid identification; they understood and complied with the requirements

Comply with the observation age of this clinical trial: 18-59 years old (both included) adults.
The subjects themselves voluntarily agreed to participate in the study, and signed an informed consent form, and can provide legal identification; understand and abide by the requirements of the trial protocol; can participate in a half-year follow-up.
Body temperature under armpit ≤37.3 °C.

Persons with full civil capacity aged 18-59 years (both included);
The subjects themselves voluntarily agreed to participate in the study, and signed an informed consent form, and can provide valid identification; understand and comply with the requirements of the trial protocol;
Body temperature under armpit ≤37.3°C;

Eligible participants in both trials were healthy population aged 18 years or older, who signed the informed consent, had negative HIV and SARS-CoV-2 (IgG and IgM) antibody testing results confirmed by rapid test kit using fingertip blood. Besides, a negative throat swab for SARS-CoV-2 nucleic acid, a clear chest CT image free from imaging features of

Male or female subjects of ≥ 18 years old with body mass index (BMI) ≥ 18.5 and ≤ 30 at the Screening Visit.
The subject can provide with informed consent and signs and dates a written informed consent form (ICF) prior to the initiation of any trial procedures.

Aged 18 years and above.
Able to understand the content of informed consent and willing to sign the informed consent.
Able and willing to complete all the secluded study process during the whole study follow-up period (about 7 months).

(1) Adults 18-85 years of age, either man or woman, who have signed the informed consent voluntarily;
(2) Confirmed as COVID-19: positive results of throat swab or blood samples by real-time RT-PCR assay for 2019-nCoV;

1. 18 to 75 years of age, male or female, willing to sign informed consent;
2. Tested positive for novel coronavirus infection after the onset of symptoms using a real time polymerase chain reaction (RT-PCR)-based diagnostic assay;
3. At least 2 viral nucleic acid tests are still positive under current antiviral therapy;

suspected and confirmed cases who met the above-mentioned suspected COVID-19 diagnostic criteria and had mild and common symptoms;
18–80 years of age, gender-neutral;
provision of informed consent.

(1) suspected and diagnosed cases of COVID-19 meeting the diagnostic criteria mentioned previously;
(2) 18–85 years old, regardless of sex;
(3) provided informed consent.

1. Suspected and confirmed cases of pneumonia in accordance with the above new signs of coronavirus infection and the standard of clinical common diagnosis;
2. Aged 18-65 years old;
3. Signing the general informed consent form.

Patients aged 18–75 years with mild COVID-19

(1) Comply with the diagnostic criteria for general type COVID-19 in the "Diagnosis and Treatment Program for COVID-19"(trial version 5th).
(2) Agree to participate in the study and the patient or legal guardian signs the informed consent form.

Hospitalized patients with COVID-19 were confirmed by pathogenic detection viral diagnostic techniques (nucleic acid test).
Meet any one of the criteria in the severe or critical category according to the national guideline of diagnosis and treatment

1. Hospitalized patients with novel coronavirus pneumonia confirmed by pathogenic detection
2. Meet any one of the criteria for severe type: 1) Respiratory distress: RP >= 30/min; 2) At rest, the oxygen saturation <= 93%; 3) Arterial partial pressure of oxygen (PaO2)/Fraction of

1. Patients diagnosed with critically ill COVID-19. It complies with the COVID-19 Critical and Critical Diagnostic Standards, namely “Pneumonitis Diagnosis and Treatment Scheme for Novel Coronavirus Infection (Trial Version 5);
3. Sign written informed consent. Patients who cannot sign informed consent must obtain

1. Severe patients diagnosed with novel coronavirus pneumonia (COVID-19);
2. Aged 35 years old to 74 years old;
3. Patients or authorized family members volunteered to participate in this study and signed informed consent. Note: Case definition: Severe NCP: Refer to the "New Coronavirus

in the study of moderate COVID-19:
male and nonpregnant female;
SARS-CoV-2 tested positive by a quantitative reverse-transcriptase polymerase-chain-reaction (qRT-PCR) assay or IgM / IgG Enzyme-Linked ImmunoSorbent Assay (ELISA);

1. Age 18 to 75 Yearsmale or female subjects
2. Diagnosed with COVID-19,according to "New Coronavirus Infected Pneumonia Diagnosis and Treatment Scheme (Pilot Sixth Edition);
3. Sever COVID-19, except shock and ICU care with organ failure;

1. Aged >=18 years at time of signing Informed Consent Form;
2. Patients with confirmed cases meeting the following criteria (1) Epidemiology history; (2) Clinical manifestations (epidemiological history and in accordance with any of the following 2 items; no clear epidemiology history, conforms to 3 items in clinical manifestations): Fever

(1) critical diagnostic criteria for severe COVID-19
(2) aged between 20 and 80 years
(3) stable condition and conscious and cooperative during the examination
(4) willing to participate in the trial and sign the informed consent form

(1) Meet NCP critical diagnosis criteria;
(2) Volunteer to participate in this study, cooperate with examination and treatment;
(3) The selected patients were basically stable, conscious and cooperative;
(4) No serious mental disorder;

(1) Patients ranging in age from 18 to 80 years (including boundary values) with no gender limitation
(2) Positive SARS-CoV-2 RNA detection from respiratory droplets by real-time polymerase chain reaction (RT-PCR)

(1) Age 18 to 80 years old (including threshold), sex unlimited, inpatient
(2) Respiratory specimen were positive for COVID-19 nucleic acid by RT-PCR;
(3) Clinical classification is mild or moderate type (with symptoms of fever, respiratory tract, imaging findings of pneumonia);

1) compliance with the diagnostic criteria for general type COVID-19 in the NHC-NATCM-China guidelines (version 6.0);
2) 18 ≤ aged ≤ 75 years;
3) Acceptance to participate in the trial by the patient, the legal guardian, or the person in

(1) Comply with the diagnostic criteria for general type COVID-19 in the "Diagnosis and Treatment Program for COVID-19"(trial version 6th).
(2) Aged >= 18 years.
(3) Agree to participate in the trial, and the patient, the legal guardian or the person in

(i) age of 18–80 years
(ii) outpatients undergoing gastroscopy
(iii) American Society of Anesthesiologists (ASA) physical status classification 1 and 2 patients at low risk of pulmonary aspiration

Patients undergoing painless gastroscopy in our hospital.

(1) 确诊为 COVID-19 普通型、重型，符合 《新型冠状病毒肺炎诊疗方案 (试行第六版) 》诊断标准，男女不限;
(2) 年龄≥18 岁且≤80 岁;
(3) 研究前详细了解试验性质、意义及可能的获益、不便、潜在危险，自愿签署知情同意书。

1. Real-time fluorescent RT-PCR detection of new coronavirus nucleic acid positive, or viral gene sequencing is highly homologous with known new coronavirus;
2. Confirmed as a novel coronavirus pneumonia common and severe patients, meets the diagnostic criteria of the "novel coronavirus pneumonia diagnosis and treatment plan (trial

All patients were confirmed as COVID-19 positive by either a reverse-transcriptase-polymerase-chain-reaction (RT-PCR) assay or clinical diagnosis according to the Guidelines for the Prevention, Diagnosis, and Treatment of Pneumonia Caused by COVID-19 (version 5) issued by the National Health Commission of the People's Republic of China.

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1. Novel coronavirus pneumonia is diagnosed met the fifth edition of the national health and Health Committee;
2. Respiratory rate in calm state >=30 times / min; OR Finger pulse oxygen saturation under rest and non oxygen absorption state <=93%; OR PO2/FiO2)<=300mmHg; OR

1. Age of 18-85 years, either male or female;
2. Diagnosed as positive for severe acute respiratory syndrome coronavirus 2 (SARS-CoV-2);
3. Symptomatic. Mild (mild clinical symptoms without signs of pneumonia on chest X-ray)

(1) Patients with Novel coronavirus pneumonia diagnosed according to <diagnosis and treatment scheme for pneumonia of new coronavirus infection (trial version 5)>;
(2) Inpatients aged >18 years old;
(3) Patients Know the informed consent, voluntary participate in this clinical trial and sign

Participants were aged 18–75 years, discharged from one of the participating hospitals after inpatient treatment for COVID-19, and had modified British Medical Research Council (mMRC) dyspnoea score of 2–3.

1. Discharged patients with COVID-19 pneumonia;
2. With dyspnea symptom (mMRC 2~3);
3. Aged 18~75 years old;
4. Have and can use smart phone (study subjects or family members);

hospitalised patients who met the following criteria:
(1)age ≥18 years;
(2) diagnosis of COVID-19 in accordance with “The Diagnosis and Treatment Scheme of COVID-19 (7th Trial Version)”;

1. Aged >= 18 years, male or female.
2. Patients who were diagnosed as COVID-19 according to the The Diagnosis and Treatment Scheme of COVID-19(The 7th Trial Edition).
3. The condition meets the discharge standards stipulated in the The Diagnosis and

Healthy adults aged 18-59 years;
informed consent;
good compliance;
axillary temperature <37.3 ℃

1. Healthy adults aged 18-59 who can provide the legal identification of the subjects, both male and female;
2. The subjects knew the content of the informed consent and the vaccination situation, voluntarily signed the informed consent, and had the ability to use the thermometer, scale

a. diagnosed with COVID-19 according to the Diagnosis and Clinical Management of 2019 novel coronavirus infected pneumonia (trial version 6 or trial version 7);
b. score of PCL-C≥50;
c. willing to participate in the study

1. According to the diagnosis and clinical management of New Coronavirus infection in 2019 (trial version 6 or trial version 7), COVID-19 was diagnosed;
2. Patients with pcl-c score >= 50;
3. Patients willing to participate in the study.

Eligible participants were airport ground staff and public security officers aged 18–59 years, without previous SARS-CoV-2 vaccination and infection, and negative for SARS-CoV-2 nucleic acid.

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1. Subjects aged 18-59 years;
2. The subjects who signed the informed consent form and participated voluntarily;
3. Subjects with good compliance;
4. Subjects with the interval of more than 14 days from the last vaccination;

1. Subjects aged 18-59 years;
2. The subjects who signed the informed consent form and participated voluntarily;
3. Subjects with good compliance;
4. Subjects with the interval of more than 14 days from the last vaccination;

(1) close contacts of COVID-19 cases, confirmed by a flow survey;
(2) at least 12 years of age, male or female;
(3) voluntarily signing a written informed consent form before the study began.

1. Confirmed as a close contact with the novel coronavirus pneumonia through flow investigation;
2. Over 12 years old (including 12 years old), no gender limit;
3. Voluntarily sign a written informed consent before the start of the study.

Eligible participants were younger adults aged between 18 and 59 years and older adults over 60 years without a history of either traveling in moderate-to-high risk areas or of contact with confirmed, asymptomatic, or suspected COVID-19 cases.

Subjects who comply with the following criteria will be enrolled:
1. Willing to participate in the study with informed consent;
2. Health male or female adults (i.e. aged 18 years and above);
3. Without a history of traveling or residence in domestic areas of high and moderate

Eligible participants were healthy adults aged over 18 YOA, with a body-mass index of 18–28 kg/m2, without a history of travelling in moderate to high-risk areas or a history of contact with confirmed, asymptomatic, or suspected COVID-19 patients.

Subjects who comply with the following criteria will be enrolled:
1. Willing to participate in the study with informed consent;
2. Health male or female adults (i.e. aged 18 years and above);
3. BMI within 18-28 kg/m2 (including both boundaries);

① Over 18 years of age, both male and female;
② After the first diagnosis and treatment of COVID-19, the SARS-CoV-2 RNA test of respiratory specimens such as sputum or nasopharyngeal swabs, has been negative for two consecutive times (sampling time interval of at least 24 h), in accordance with China COVID-

COVID-19 has been diagnosed, and the nucleic acid test of respiratory specimens such as sputum or nasopharyngeal swabs has been negative for two consecutive times after treatment (sampling time interval of at least 24 hours);
The nucleic acid test of specimens such as sputum, throat swabs, blood, feces and other

(i) age of ≥18 years;
(ii) body mass index (BMI) between 18.0 and 26.0 kg/m2,
(iii) healthy status as evaluated by previous medical history, physical examination, 12-lead electrocardiogram, chest CT scan, and laboratory tests;

Able to provide written informed consent
Males or females. Aged ≥ 18 years old
Body mass index (BMI) between 18.0 and 26.0 kg/m2
Normal or abnormal but non-clinical significant physical examination, vital signs, 12-ECG

Eligible participants were healthy adults aged 18–45 years as defined by the inclusion and exclusion criteria related to medical history, physical examination, and clinical laboratory tests, with no participation in any test drugs within the last three months.

Subjects are fully aware of the purpose, nature, methods and possible adverse reactions of the study, voluntarily serve as subjects, and sign the informed consent form before the start of any study procedures, and ensure that he/she will participate in any procedures in the study in person;

Patients diagnosed with mild or moderate COVID-19 (as per the Diagnosis and Treatment Protocol for Novel Coronavirus Pneumonia (Trial, 8th edition), China, or Clinical Management of COVID-19, WHO);

Has at least one COVID-19 symptom, e.g., fever, cough, shortness of breath, sore throat,

(1) Healthy subjects aged 3~17 years old;

(2) The subjects/guardians are able to understand and sign the informed consent voluntarily (both subjects and guardians should sign the informed consent for those aged 8~17 years old);

Healthy children and adolescents aged 3-17 years;

The subject and/or guardian can understand and voluntarily sign the informed consent form (double sign required for 8-17 years old);

Proven legal identity.

Aged 18 to 55 years old in MID cohort; aged 6-17 years old in MIN cohort; aged 56 years and above in OLD cohort;

Able to understand the content of informed consent and willing to sign the informed consent;

Able and willing to complete all the secluded study process during the whole 6 months study

Aged 18 to 55 years old in MID group; aged 6-17 years old in MIN group; aged 56 years and above in OLD group; aged in EBO group is not limited.

Able to understand the content of informed consent and willing to sign the informed consent

Able and willing to complete all the secluded study process during the whole 6 months study

1. Health subjects aged 18-59 years, who have been completed two-dose regimen of inactive SARS-CoV-2 vaccine in the past 3-6 months, or received one dose of inactive SARS-CoV-2 vaccine in the past 1-3 months.

2. The subject can provide with informed consent and sign informed consent form (ICF).

Health subjects aged 18-59 years, who have been completed two-dose regimen of inactive SARS-CoV-2 vaccine in the past 3-6 months, or received one dose of inactive SARS-CoV-2 vaccine in the past 1-3 months.

The subject can provide with informed consent and sign informed consent form (ICF).

1. Get adequate knowledge of the trial objective, and general knowledge of the pharmacological actions of the investigational drug, and potential adverse reactions; voluntarily sign the informed consent form in accordance with the principles of the Declaration

A signed informed consent form (ICF) from the patient or their legally authorized representative.

Healthy males or females, ages ≥18 to ≤45 years. Male subjects with weight ≥50 kg, or

1. Healthy adults, or adults with pre-existing but stable medical conditions (Participants didn't require significant change in therapy or hospitalization within 3 months before enrollment).

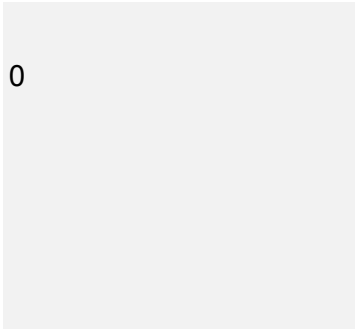
2. Participants who have received priming vaccination by two doses of inactivated whole-

Age ≥ 18 years old,
Through asking medical history and physical examination, the investigator judged that the health condition is good: the chronic disease is stable and there is no change within 3 months of medication;

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Deviation in Exclusion Criteria (1 if Yes, 0 if not specified in one or more of data sources)
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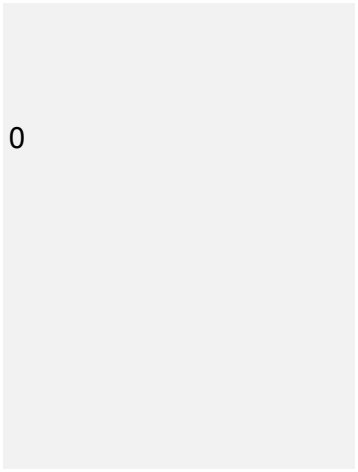
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Exclusion Criteria

Exclusion criteria included a physician decision that involvement in the trial was not in the patient's best interest, presence of any condition that would not allow the protocol to be followed safely, known allergy or hypersensitivity to lopinavir–ritonavir, known severe liver disease (e.g., cirrhosis,

1. Any situation that makes the programme cannot proceed safely;
2. Known allergy or hypersensitivity reaction to lopinavir / ritonavir;
3. Increase in alanine aminotransferase (ALT) / aspartate aminotransferase (AST) is more than 5 times the upper limit of normal;
4. Use of medications that are contraindicated with lopinavir / ritonavir and that cannot be replaced

(1) were pregnant or breastfeeding women; (2) had aspartate aminotransferase (AST) or alanine aminotransferase (ALT) >5× upper normal limit, creatinine clearance <50 ml/min (Lu and Dong, 2019); (3) were allergic or intolerant to therapeutic drugs; (4) were HIV-positive patients; (5) had

(1) Patients are diagnosed as a case of severe NCIP;
(2) Patients are pregnant or breastfeeding women;
(3) Patients have aspartate aminotransferase (AST) or alanine aminotransferase (ALT) > 5Ã—UNL or creatinine clearance <50 ml / min;
(4) Patients are allergic or intolerant to the proposed antiviral therapeutic drugs;

Patients were excluded if known life expectancy was 48 h or less, on home oxygen at baseline, pregnant or lactating, diagnosed with end-stage diseases, or having used immunosuppressants for 6 months or longer.

1) Severe basic diseases that affect survival, including: uncontrolled malignant tumors with multiple metastases that cannot be removed, hematological diseases, cachexia, active bleeding, severe malnutrition, HIV, etc.;
2) Obstructive pneumonia caused by lung tumors, severe pulmonary interstitial fibrosis, alveolar proteinosis, allergic reaction alveolitis;

Not specified

(1) any situation in which the program cannot proceed safely;
(2) allergic constitution;
(3) serious basic diseases affecting survival, including uncontrolled malignant tumors with multiple metastases that cannot be resected, blood diseases, malignant fluid, active bleeding, severe malnutrition, HIV, etc.; The main organ function is abnormal

(I) immunodeficiency, immunosuppressive drugs, or glucocorticoids within the past 3 months; (II) preparing for pregnancy, pregnant women, or lactating women; (III) allergies to more than two types of drugs or foods or allergies to ingredients of the trial drug; (IV) psychosis or those without self-knowledge ability; (V) estimated survival time less than 48 h from the start of screening; (VI) tracheal

(1) Immunodeficiency diseases, or those using immunosuppressive agents or glucocorticoids within the past 3 months;
(2) Pregnancies, pregnant women and lactating women;
(3) An allergic condition, such as a history of allergies to two or more drugs or foods, or a known allergy to the ingredients in this test);

We excluded patients if they presented with any condition that would not allow the protocol to be followed safely; had a history of allergy or hypersensitivity to interferons or any of the ingredients used in this trial; had a history of myocardial infarction and other serious cardiovascular diseases; were unable to receive nebulized compound; and/or voluntarily requested to withdraw from the trial.

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1. Pregnant or lactating women;
2. Patients who cannot be inhaled by atomization;
3. Patients who are intolerant or allergic to interferon preparations;
4. Other patients were judged unsuitable for inclusion in the trial by researchers.

Not specified

1. Intubation required immidiately in case of cardiac arrest or asphyxia;
2. Pregnancy, severe chronic respiratory, uncontrolled gastric reflux disease and any nasopharyngeal anatomical obstacle;
3. Those patients has intubated in ICU.

(1) pregnancy or lactation; (2) immunoglobulin allergy; (3) IgA deficiency; (4) preexisting comorbidity that could increase the risk of thrombosis; (5) life expectancy less than 24 hours; (6) disseminated intravascular coagulation; (7) severe septic shock; (8) PaO2/FIO2 of less than 100; (9) severe congestive heart failure; (10) detection of high titer of S protein–RBD-specific (receptor binding

1.The doctor believes that the patient is not suitable to participate in this trial, including those who may not cooperate, do not comply with the requirements of the procedure, or participate in this trial may put the patient in an unsafe situation.
2.Pregnant or lactation periods women;
3.Immunoglobulin allergy;

1) known or suspected allergy to the composition of azvudine tablets;
2) patients with malabsorption syndrome or any other condition that affects gastrointestinal absorption, the need for intravenous nutrition or an inability to take oral medication;
3) patients on anti-HIV treatment;

(1) known or suspected allergy to the components of Azivudine tablets;
(2) patients with malabsorption syndrome or any other condition that affects gastrointestinal absorption and requires intravenous nutrition or unable to take oral medication;
(3) patients who is currently receiving anti-hiv treatment;
(4) patients with one of the following conditions: respiratory failure and mechanical

Exclusion criteria were age below 18 years; severe conditions including malignancies, heart, liver, or kidney disease or poorly controlled metabolic diseases; unsuitability for oral administration; pregnancy or lactation; allergy to hydroxychloroquine; inability to cooperate with investigators due to cognitive impairments or poor mental status; severe hepatic impairment (for example, Child Pugh

1. Aged less than 18 years old;
2. Other serious medical diseases such as malignant tumor, heart, liver and kidney disease, uncontrollable metabolic disease, etc;
3. Not suitable for gastrointestinal administration;
4. Pregnant or lactating women;

Patients were ineligible if they had been diagnosed with any kind of cancers or the doctor declared the situation to belong to the critically severe condition. We excluded patients who were participating in other clinical trials or who have participated in other clinical trials within 3 months.

Patients were ineligible if they were pregnant, had been diagnosed as having cancer, or were experiencing critical respiratory failure.

1. Malignant disease in the past five years;
2. The clinical prediction is that there is no hope for survival, only cases of hospice care, or cases of deep coma that do not respond to supportive treatment measures within three hours of admission;
3. Subjects who are participating in other clinical trials or within 3 months;

Patients were excluded if they had a peripheral blood leukocyte cell count greater than 15000 per μL , any comorbidity (eg, chronic obstructive pulmonary disease, hypertension, diabetes, coronary heart disease, and/or malignancy), required invasive ventilation, developed shock or other organ failure that required admission to an intensive care unit, had an allergy to rhG-CSF, or were

1. Combining basic diseases such as hypertension, diabetes, and coronary heart disease;
2. Malignant tumor or with a history of malignant tumor;
3. pregnant or lactating women;
4. suffer from severe mental illness;
5. History of unstable angina pectoris or myocardial infarction within 6 months; coronary

Patients were excluded from consideration in the study if they had received systemic antiviral and immune regulation within 3 months; viral RNA nucleic acid test has turned negative after diagnosis; allergic to M. vaccae; had other severe disease such as severe heart disease, myocardial damage, significant vascular sclerosis, endocarditis, extremely weak, and severe anemia; pregnant women

1. Pregnant women;
2. Patients with a history of angina pectoris, myocardial infarction and other serious cardiovascular diseases;
3. The researchers judged that this study was not in the interests of patients.

Not specified

1. Patients with presence, coagulopathy or hypotension;
2. Pregnant and lactating women;
3. People with allergies or allergies to Xuebijing Injection and its components;
4. Severe basic diseases that affect survival, including: uncontrolled malignant tumors that have metastasized and cannot be removed, blood diseases, cachexia, active bleeding, severe

(1) cases of severe vomiting that are difficult to take pills or cases that have difficulties in absolving the medicine after oral administration;
(2) pregnant women and lactating women;
(3) subjects received specific antiviral medications such as lopinavir/lidonavir, ribavirin, monoclonal

(1) presence of any condition that would not allow the protocol to be followed, including known allergy to leflunomide, use of medications that are contraindicated with leflunomide and that could not be replaced or stopped during the trial period;
(2) pregnant or breast-feeding;

1. severe vomiting and difficulty in taking oral medication or absorbing in oral medication;
2. pregnant and nursing women;
3. subjects received lopinavir/ridonavir, ribavirin, monoclonal antibodies and other specific antiviral drugs in the week prior to admission;

The exclusion criteria were patients who were clearly diagnosed with a psychiatric disorder, including depression, bipolar disorder, etc, in the 6 months prior to their diagnosis of COVID-19; patients with psychotic symptoms; patients with HAMD17 score≥24 or HAMA score≥21; patients with a high risk of suicide, defined as having a history of attempts by suicide in the 6 months prior to

1. In the six months before the diagnosis of 2019-nCoV acute respiratory disease, the patient was clearly diagnosed as a psychiatric disorder, including depression, bipolar disorder and so on;
2. Patients with psychotic symptoms;
3. A history of suicide attempt in the 6 months before the study, or a score of, more than 3 on item 3

1 The patient had a history of allergy to XBJ injection. The allergic reactions to XBJ injection were rash and anaphylactic shock.
2 Pregnant women or breastfeeding mothers.
3 Severe underlying disease, malignant tumor, hematopathy or AIDS included

1. Those who are allergic to Xuebijing injection and its components;
2. Pregnant or lactating women;
3. Serious basic diseases affecting survival, including: malignant tumor, hematopathy, HIV, etc;
4. Expected death within 48 hours;
5. Patients who are participating in other clinical trials;

①Allergic to favipiravir or tocilizumab;
②Pregnant or lactating woman;
③ALT or AST> 5 times of upper limit of normal;

Cases of severe vomiting that make it difficult to take the drug orally

Allergic to Favipiravir or tocilizumab

Pregnant and lactating women

Subjects received specific antiviral drugs such as lopinavir / ritonavir, ribavirin, arbidol, chloroquine phosphate, hydroxychloroquine, and monoclonal antibodies within one week before admission

8.2 Exclusion criteria for the first dose

Confirmed, suspected, or asymptomatic COVID-19 cases;

Those with positive antibody tests of the COVID-19;

History of SARS virus infection (identified through self-report or on-site inquiry)

Exclusion criteria for the first dose

- Confirmed, suspected, or asymptomatic COVID-19 cases;
- Those with positive antibody tests of the COVID-19;
- History of SARS virus infection (identified through self-report or on-site inquiry)

1. Confirmed cases, suspected cases or asymptomatic cases with COVID-19 (refer to Information System of China Disease Prevention and Control);
2. COVID-19 antibody test positive;
3. Fever (armpit temperature > 37.0 degree C), dry cough, fatigue, nasal obstruction, runny nose,

Exclusion criteria were a history of travelling to Hubei Province (China), regions outside of China, or regions with reported COVID-19 cases from December, 2019; a history of infection with SARS-CoV; fever, cough, runny nose, sore throat, diarrhoea, dyspnoea, or tachypnoea in the 14 days before vaccination; abnormalities in laboratory tests; pregnancy or lactation; allergy to any ingredient

Exclusion criteria were a history of travelling to Hubei, regions outside of China, or regions within China with any reported COVID-19 cases since December, 2019;

a history of infection with SARS-CoV; fever (axillary temperature more than 37·3°C if aged older than 14 years, and axillary temperature of more than 37·5°C if aged 14 years or younger),

1. Confirmed cases, suspected cases or asymptomatic cases with COVID-19 (refer to Information System of China Disease Prevention and Control);
2. Positive in serum antibodies (IgG and IgM) screening of COVID-19;
3. Has a history of SARS virus infection (self-reported, site information);
4. Fever (armpit temperature > 37.0 degree C), dry cough, fatigue, nasal obstruction, runny nose

1. Have had any acute illness, as determined by the investigator, with or without fever, within 72h prior to the prime vaccination. An acute illness which is nearly resolved with only minor residual symptoms remaining is allowable if, in the opinion of the investigator, the residual symptoms will not compromise their well-being if they participate as trial subjects in the trial, or that could prevent, limit,

For adult group (age ≥18 and ≤55)

Have had any acute illness, as determined by the investigator, with or without fever, within 72 hours prior to the prime vaccination. An acute illness which is nearly resolved with only minor residual symptoms remaining is allowable if, in the opinion of the investigator, the residual symptoms will not

Individuals with confirmed cases, suspected cases, or asymptomatic cases of COVID-19 as referred to in the Information System of China Disease Prevention and Control were excluded. Those who had close contact with confirmed or suspected cases or had a history of travel to a foreign or domestic epidemic community within the 14 days before vaccination were also excluded. To be

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1. Within 14 days before vaccination, subjects have been abroad and to villages/communities experienced COVID-19 epidemics, and in contact with COVID-19 cases or suspected cases. Subjects are under isolation observation, or living in the villages/communities with COVID-19 cases or suspected cases;

2. Confirmed cases, suspected cases or asymptomatic cases with COVID-19 (refer to Information

1. Within 14 days before vaccination, subjects have been abroad and to villages/communities experienced COVID-19 epidemics, and in contact with COVID-19 cases or suspected cases. Subjects are under isolation observation, or living in the villages/communities with COVID-19 cases or suspected cases;

2. Confirmed cases, suspected cases or asymptomatic cases with COVID-19 (refer to Information

Individuals with confirmed cases, suspected cases or asymptomatic cases of COVID-19 as referred to the Information System of China Disease Prevention and Control were excluded. Those who had close contact with confirmed or suspected cases, or had travel history to a foreign or domestic epidemic community within 14 days before vaccination were also excluded. To be included,

1. Within 14 days before vaccination, subjects have been abroad and to villages/communities experienced COVID-19 epidemics, and in contact with COVID-19 cases or suspected cases. Subjects are under isolation observation, or living in the villages/communities with COVID-19 cases or suspected cases;

(1) previously diagnosed with depression and currently taking medication,
(2) had prior cognitive dysfunction and
(3) had experienced another major stressful event (e.g., divorce, bereavement) in the past year.

1. Previously diagnosed with depression and currently taking medication
2. Prior cognitive dysfunction
3. Experienced another major stressful event (e.g. divorce, bereavement) in the past year

(1) pregnant or lactating women;
(2) respiratory failure necessitating mechanical ventilation;
(3) liver failure (total bilirubin 10 mg/dL and/or severe coagulation disorders);
(4) renal function failure [urine 0.5 m•kg-1•h-1, creatinine (Cr) or blood urea nitrogen (BUN) 1.5×

Age < 14 years or > 80 years;

Pregnant or lactating female;

One of the following items occurred at the enrollment: (i) respiratory failure necessitating mechanical ventilation; (ii) liver failure: total bilirubin ≥ 10mg/dL and/or severe coagulation disorders; (iii) renal function failure; although adequate circulating blood and cardiac output, urine < 0.5ml/kg·h, Cr or

hypersensitivity to darunavir, cobicistat, or any excipients;

patients with severe liver injury (Child-Pugh Class C);

patients receiving concomitant medications that are highly dependent on cytochrome P450 3A clearance, and for which the elevated plasma concentrations are associated with serious or life-

Hypersensitivity to darunavir, cobicistat, or any excipients

Patients with severe liver injury (Child-Pugh Class C)

Concomitant medications that are highly dependent on CYP3A clearance, and the elevated plasma concentrations are associated with serious or life-threatening events.

Subjects were considered to be unable to complete the study, or not suitable for the study by

1) known or suspected to be allergic to LPV/r or arbidol;

2) having severe nausea, vomiting, diarrhea or other complaints affecting oral intake or absorption in the digestive tract;

3) taking other drugs that may interact with LPV/r or arbidol;

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Have a clear history of lopinavir or ritonavir or arbidol allergy
Severe nausea, vomiting, diarrhea and other clinical manifestations affect the oral or absorption of the drugs
At the same time, take drugs that may interact with lopinavir or ritonavir or arbidol
Patients with serious underlying diseases, including but not limited to heart disease (including

Exclusion criteria included pregnancy or breast feeding; hepatic cirrhosis; alanine aminotransferase or aspartate aminotransferase more than five times the upper limit of normal; known severe renal impairment (estimated glomerular filtration rate <30 mL/min per 1·73 m2) or receipt of continuous renal replacement therapy, haemodialysis, or peritoneal dialysis; possibility of transfer to a non-

Physician makes a decision that trial involvement is not in patients' best interest, or any condition that does not allow the protocol to be followed safely.
Severe liver disease (e.g. Child Pugh score ≥ C, AST>5 times upper limit)
Pregnant or breastfeeding, or positive pregnancy test in a predose examination
Patients with known severe renal impairment (estimated glomerular filtration rate <30 mL /min/1.73

1 对氯喹、羟氯喹过敏的患者；2 妊娠期妇女；3 合并心、肺、肾、脑、血液等重要器官严重疾病伴功能不全的患者；4 视网膜疾病、听力减退或听力丧失的患者；5 严重神经或精神疾病的患者；6 研究者认为无法按要求完成研究或不适宜参与研究者

Hypersensitivity to chloroquine or hydroxychloroquine;
Women during pregnancy;
Severe heart, lung, kidney, brain, blood diseases or other important systemic diseases;
Participants with retinal disease, hearing loss;
Participants with severe neurological and mental illness;

Exclusion criteria were allergy to vitamin C, pregnancy or breastfeeding, expected survival duration < 24 h, and previous history of glucose-6-phosphate dehydrogenase deficiency or end-stage pulmonary disease. Patients who were already enrolled in other clinical trials were excluded as well.

Allergic to vitamin C;
Dyspnea due to cardiogenic pulmonary edema;
Pregnant or breastfeeding;
Expected life is less than 24 hours;
There is a state of tracheotomy or home oxygen therapy in the past;

Exclusion criteria included severe immunosuppression (human immunodeficiency virus infection and long-term use of immunosuppressive agents), pregnant or breastfeeding women, corticosteroid needed for other diseases, refractory hypertension, epilepsy or delirium, glaucoma, active gastrointestinal bleeding within 3 months, refractory hypokalemia, secondary bacterial or fungal

severe immunosuppression (HIV infection, long-term use of immunosuppressive agents
pregnant or lactation period women
glucocorticoids are needed for other diseases
unwilling or unable to participate or complete the study
participate in other study

(1) alanine aminotransferase ≥ 5 × the upper limit of normal (ULN), total bilirubin level ≥ 3× ULN, or creatinine ≥ 1.5 × ULN;
(2) severe liver disease;
(3) severe or critical cases;

ALT≥5 times of ULN, level of total bilirubin≥3 times of ULN, or Cr≥1.5 times of ULN.
Patients with serious severe liver disease.
Excluded who diagnosed as pneumonia patients with novel coronavirus infection of severe type and critical type. 【Diagnostic criteria reference the official guideline "Pneumonia Diagnosis and Treatment Scheme for Novel Coronavirus Infection (Trial Version 5) "】

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The exclusion criteria included patients with shock or COVID-19 combined with any one of other organ failures, those who received invasive ventilation, or patients with any malignant tumor, pregnancy or breastfeeding, or co-infection of other pathogens.

Not Specified

Pregnancy, lactation and those who are not pregnant but do not take effective contraceptives measures;
Patients with malignant tumor, other serious systemic diseases and psychosis;
Patients who are participating in other clinical trials;

'-Family history of seizure, epilepsy, brain or mental disease
- Participant that has an allergic history to any ingredient of vaccines
- Woman who is pregnant, breast-feeding or positive in pregnancy test on day of enrollment, or is planning to be pregnant during the next 6 months

Family history of seizure, epilepsy, brain or mental disease
Subject allergic to any component of the investigational vaccine, or a more severe allergic reaction and history of allergies in the past.
Woman who is pregnant, breast-feeding or positive in β-HCG (human chorionic gonadotropin) pregnancy test (urine) on day of enrollment, or become pregnant during the next 6 months

(1) Travel / residence history of Wuhan city and surrounding areas or other communities with case reports within 14 days prior to the entry;
(2) Contact with SARS-CoV-2 infected persons (positive for nucleic acid detection) within 14 days prior to the netry;

Travel history / residence history of Wuhan city and surrounding areas, or other communities with case reports within 14 days;
History of contact with a SARS-CoV-2 infection (positive in nucleic acid test) within 14 days;
Have contacted patients with fever or respiratory symptoms from Wuhan and surrounding areas, or from communities with case reports within 14 days;

(1) Travel / residence history of Wuhan city and surrounding areas or other communities with casereports within 14 days prior to the entry;
(2) Contact with SARS-CoV-2 infected persons (positive for nucleic acid detection) within 14 days prior to the netry;

Travel / residence history of Wuhan city and surrounding areas or other communities with case reports within 14 days prior to the enrolment;
Contact with SARS-CoV-2 infected persons (positive for nucleic acid detection) within 14 days prior to the enrolment;
Contact patients with fever or respiratory symptoms from Wuhan city and surrounding areas, or from

18-59

(1) Having traveled to or lived in Wuhan or surrounding areas or communities where confirmed cases have been reported within the previous 14 days;
(2) Having contact with patients who were infected with COVID-19 (who have tested positive for

Exclusion Criteria:

Travel history / residence history of Wuhan city and surrounding areas, or other communities with case reports within 14 days;

Exclusion Criteria:

Travel / residence history of Wuhan city and surrounding areas or other communities with case reports within 14 days prior to the enrolment;

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Not specified

Not specified

Phase Ia:

Contraindications for vaccination.
History of allergy to vaccines or drugs.
Immunization with any vaccine within 1 month

Not specified

Excluded for novel coronavirus (SARS-CoV-2) infection

Having one of the following evidence on SARS-CoV-2 infection:

SARS-CoV-2 determined by reverse transcription-polymerase chain reaction (RT-PCR) and/or next

Phase I

1) The vital signs, physical examination and laboratory test indexes specified in the protocol were abnormal, which were judged by clinicians to be of clinical significance;
The vital signs, physical examination and laboratory test indicators of the population specified in the plan are abnormal and have clinical significance as determined by the clinician;
Have a history of severe allergies to any component of the research vaccine, including aluminum preparations, such as: anaphylactic shock, allergic laryngeal edema, allergic purpura, thrombocytopenic purpura, local allergic necrosis (Arthus reaction); or any previous History of
The vital signs and physical examination results of the population specified in the plan have clinical significance as determined by the clinician;
A history of severe allergies to any component of the test vaccine, including aluminum preparations, such as: anaphylactic shock, allergic laryngeal edema, allergic purpura, thrombocytopenic purpura, local allergic necrosis (Arthus reaction), dyspnea, vascular neuropathy Edema, etc.; or any previous

Pregnant and breastfeeding women, people with allergic history, convulsion, epilepsy, encephalopathy, mental illness, acute infection, severe and uncontrollable cardiovascular disease or other chronic illnesses were excluded.
Exclusion criteria of prime dose:

Subjects with a medical or family history of convulsions, epilepsy, encephalopathy, and psychosis.
Allergic to any ingredient in the study vaccine, or used to have a serious vaccine allergic reaction.
First dose exclusion criteria:

Positive serum immunoglobulin M (IgM) and IgG to the SARS-CoV-2.
A Known History of HIV infection
(1) Allergic constitution, known to be allergic to baloxavir marboxil or favipiravir or pharmaceutical excipients;
(2) Weight < 40 kg;
(3) Critical illness meeting one of the following conditions: respiratory failure and mechanical

1. Allergic constitution, known to be allergic to balosavir or farpiravir or pharmaceutical excipients;
2. Body weight <40 kg;
3. Considered as severe disease: has an ongoing respiratory deficiency and subjected to invasive mechanical ventilation; or presence of shock; or admitted to the ICU due to complications;

Patients excluded from the study on the basis of the following criteria:
clear evidence of bacterial infection;
severe primary diseases of the heart, kidney, lung, endocrine, blood, metabolism, and gastrointestinal tract that may affect the patients' participation in the trial or the outcome of the study,

(1) clear evidence of bacterial infection;
(2) severe primary diseases, such as heart, kidney, lung, endocrine, blood, metabolism, or gastrointestinal tract diseases, which may affect the patient's participation in the trial or affect the outcome of the study;

1. Clear evidences?of bacterial infection;
2. Serious primary diseases judged by researchers,such as heart, kidney, lung, endocrine, blood, metabolism and gastrointestinal tract,?may affect the patients' participation in the trial or the outcome of the study;

could not guarantee that medication would be taken during the trial observation period;
more than 7 days after diagnosis;
severe primary respiratory diseases;
pregnant or planning pregnancy;

(1) Patients who can not guarantee compliance of using TCM during the treatment period, or patients who are difficult to take medicine by oral or nasal route.
(2) Patients who have been more than 7 days since definite diagnosis.
(3) Patients with severe primary respiratory disease or other pathogenic microbial pneumonia that

1 Pregnant or lactating women
2 Allergy to herbs of Shenhuang Granule
3 History of a severe primary disease, including unresectable tumors, blood diseases (such as acute leukemia and purpura hemorrhagica) , HIV, severe liver dysfunction (serum bilirubin level > 205.2

1. Pregancy or lactating women;
2. People with allergies or allergies to components of Truncated Torsion formula;
3. Patients with serious basic diseases such as malignant tumor, cirrhosis, chronic renal failure (uremic stage), hematological disease, HIV, etc.;

(1) Patients who are participating in other clinical trials;
(2) Pregnant or breastfeeding women;
(3) There are other life-threatening diseases such as cancer;
(4) Expected survival time <24 h;

1. Involving in other drug clinical trial;
2. Pregnant or breastfeeding;
3. Presence of other serious diseases that may be life threatening, such as cancer;
4. Expected life is less than 24 hours;

a known allergy to 4-aminoquinolines drugs or analogs;
any comorbidities including hematological diseases, mental illnesses, retinopathy, maculopathy, severe liver diseases, hypokalemia, hypomagnesemia, pancreatitis, severe renal impairment, and glucose-6-phosphate dehydrogenase deficiency;

1 Patients with identified allergies to 4-aminoquinoline and lopinavir/litonavir;
2 Patients with server diseases of the blood system;
3 Patients with identified severe liver disease, underlying cirrhosis or alanine aminotransferase (ALT)/aspartate aminotransferase (AST) elevation more than 3 times the normal upper limit;

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1. Patients with identified allergies to 4-aminoquinoline and lopinavir/litonavir;
2. Patients with server diseases of the blood system;
3. Patients with identified severe liver disease, underlying cirrhosis or alanine aminotransferase (ALT)/aspartate aminotransferase (AST) elevation more than 3 times the normal upper limit;

(1) complications due to other underlying severe diseases, such as chronic obstructive pulmonary disease, obstructive pulmonary disease, coronary heart disease, and hypertension,
(2) serious mental illness,
(3) cognitive dysfunction preventing an understanding of the trial process and rehabilitation content,

1. Patients with other serious diseases such as lung diseases, cardiovascular and cerebrovascular diseases, hematopoietic diseases, autoimmune diseases, digestive system, or mental illness;
2. Pregnant or lactating women;
3. Participate in other forms of exercise during the trial

(1) Known or suspected to be allergic to the components of the test drug.
(2) Patients with confirmed hepatitis B virus, hepatitis C virus, human immunodeficiency virus (HIV), or other viral infections.
(3) Patients classified as severe or critical types with complications such as respiratory failure and

(1) Patients with known or suspected to be allergic to the components of the test drug;
(2) Patients with known hepatitis B, C, AIDS and other viral infections;
(3) Patients with severe or severe clinical classification who have experienced complications such as respiratory failure and acute respiratory distress syndrome (ARDS);

1) critical patients;
2) patients who could not guarantee compliance of using Q-14 during the treatment period, or patients who had difficulty taking drugs via the oral or nasal route;
3) patients with severe primary respiratory disease or other pathogenic microbial pneumonia with

(1) Critical patients.
(2) Patients who can not guarantee compliance of using TCM during the treatment period, or patients who are difficult to take medicine by oral or nasal route.
(3) Patients with severe primary respiratory disease or other pathogenic microbial pneumonia that

(i) pregnant women,
(ii) patients undergoing colonoscopy at the same time
(iii) suspected or confirmed patients with COVID-19.

1. Patients undergoing general gastroscopy;
2. With trachea dysfunction;
3. pregnant women, children, age more than 80 years;
4. Patients with difficulty in intubation.

(1) 轻型、危重型 COVID-19;
(2)任何其他影响临床试验评估的呼吸道疾病;
(3) 合并哮喘，需每日治疗;
(4) 胸部 CT 证实存在严重的肺间质病变、支气管扩张等肺部疾病;

1. Patients with mild and critical novel coronavirus pneumonia;
2. Acute respiratory disease caused by non- novel coronavirus pneumonia;
3. Patients with diabetes and hypertension accompanied by severe primary immunodeficiency disease acquired immunodeficiency syndrome congenital respiratory malformation congenital heart

Main exclusion criteria included age <18 years old, pregnancy, history of arrhythmia, chronic respiratory failure caused by other diseases such as heart failure, thoracic deformity, structural lung disease, hemodynamic instability, severe immunodeficiency, recent use of immunosuppressants, allergies, estimated survival time <3 days, withdrawal from research or return visits.

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1. Aged < 7-year-old;
2. previous arrhythmia;
3. previous chronic respiratory failure caused by other diseases, such as heart failure, thoracic deformity, structural lung disease, etc.;

1. < 18 or > 85 years old;
2. Pregnancy and lactation;
3. Serious heart, liver, kidney and hematopoietic system diseases, abnormal liver or kidney function;
4. Suffering from pneumonia caused by other known viruses;

(1) Death is expected within 48 hours;
(2) Basic diseases such as primary immunodeficiency disease, acquired immunodeficiency syndrome, congenital respiratory malformation , congenital heart disease, gastroesophageal reflux disease, congenital pulmonary airway malformation and more with the exact evidence of bacterial

patients with mMRC dyspnoea of 4–5 were excluded for reasons of safety.
Other exclusion criteria were: resting heart rate over 100 bpm, uncontrolled hypertension, uncontrolled chronic disease (eg, diabetes with random blood glucose >16.7 mmol/L, haemoglobin A1C >7.0%), cerebrovascular disease within 6 months, intra-articular drug injection or surgical

1. Dyspnea scale mMRC 0~1 or 4;
2. Resting heart rate>100bpm;
3. Adjusting dosage of drugs which affect cardiopulmonary function or heart rate (such as: trimetazidine, salbutamol, beta-blocker);

(1) known or suspected allergy to the components of BFHX;
(2) acute infections caused by other viruses or bacteria;
(3) abnormal liver and kidney function tests (alanine aminotransferase [ALT], aspartate aminotransferase [AST], and serum creatinine [SCr] ≥1.5 times the upper limit of normal);

1. Known or suspected allergy to the components of BFHX.
2. Acute infections, along with other viruses or bacteria.
3. Abnormal liver and kidney function tests (ALT, AST, SCr >= 1.5 times the upper limit of normal).
4. Participating in other drug clinical trials.

Pulse <50 or >100 beats/min;
SBP≥140 mmHg or DBP≥90 mmHg; BMI <18 or >30 kg/m2;
clinical screening laboratory evaluations are 1.2 times the normal reference range
Pregnancy or lactation or be menstruating or egg or sperm donation

Subjects meeting any of the following exclusion criteria were not included in the study
1. The subjects who did not meet the health standards in the comprehensive physical examination mainly included: (1) Patients with abnormal vital signs and clinical significance (awake pulse < 50 beats / min or > 100 beats / min, systolic blood pressure >= 140 mmHg or diastolic blood pressure

a. history of mental illness;
b. inability to use the internet.

1. Patients with mental history;
2. Patients who do not use the Internet.

(1) history or family history of allergy, convulsion, epilepsy, encephalopathy or psychosis;
(2) any intolerance or allergy to any component of the vaccine;
(3) known or suspected diseases including severe respiratory disease, severe cardiovascular disease, severe liver or kidney disease, medically uncontrollable hypertension (systolic blood

1. Patients with history of allergy, convulsion, epilepsy, encephalopathy and psychosis or family history;

2. Those who are allergic to any component of the vaccine, and those who have had any history of severe allergy to the vaccine (such as acute allergic reaction, urticaria, skin eczema, dyspnea,

1. Patients with history of allergy, convulsion, epilepsy, encephalopathy and psychosis or family history;

2. Those who are allergic to any component of the vaccine, and those who have had any history of severe allergy to the vaccine (such as acute allergic reaction, urticaria, skin eczema, dyspnea,

(1) COVID-19 infection confirmed by etiological tests and clinical manifestations or signs;

(2) an allergic constitution, such as an allergy history to two or more drugs or food or a known allergy to the ingredients of this drug;

(3) pregnancy or lactation.

1. Confirmed as a confirmed case of new coronavirus pneumonia after pathogenic testing and clinical manifestations or signs;

2. Allergies, such as those with a history of allergies to two or more drugs or foods, or those who are known to be allergic to the ingredients of this drug;

a history of COVID-19 or positive COVID19 screening test (reverse transcription polymerase chain reaction or enzyme-linked immunosorbent assay [ELISA]) results;

a history of SARS, autoimmune disease, confirmed or suspected immunosuppressive or immunodeficiency disorder, known allergic reactions to vaccines or vaccine components, or any

Subjects who meet any of the following criteria will be excluded:

1. Confirmed or asymptomatic COVID-19 cases;

2. Positive SARS-CoV-2 nucleic acid test;

3. Positive SARS-CoV-2 IgG or IgM test (reference to nucleic acid test and chest CT if necessary);

participants with a history of COVID-19 or those who tested positive at screening (by RT–PCR or ELISA);

with a history of SARS, autoimmune diseases, severe allergy reactions to vaccines, congenital or acquired angioneurotic oedema;

Subjects who meet any of the following criteria will be excluded:

1. Confirmed or asymptomatic COVID-19 cases;

2. Positive SARS-CoV-2 nucleic acid test;

3. Positive SARS-CoV-2 IgG or IgM test (reference to nucleic acid test and chest CT if necessary);

① Allergic to favipiravir;

② Pregnant or lactating women;

③The researchers considered the patient was not suitable to participate in this clinical trial.

Those allergic to fapilavir;

Pregnant or lactating women;

Unstable liver, kidney, and heart diseases;

History of mental disorders, substance abuse or dependence;

(i) allergy to humanized monoclonal antibodies and any ingredient of SCTA01;

(ii) suspected or verified SARS-CoV-2 infection;

(iii) a history of severe allergies;

(iv) infection or fever within 14 days before enrollment;

Having a history of severe allergy, such as severe allergic reactions, urticaria, angioedema;

Having one of the following evidence on SARS-CoV-2 infection(previous tests were accepted) :

SARS-CoV-2 positive: reverse transcription-polymerase chain reaction (RT-PCR) and/or next

Not Specified

Health status: clinically significant histories of heart, liver, kidney, digestive tract, nervous system, respiratory system, blood and lymphatic system, immune system, mental, metabolic, and bone abnormalities.

Subjects who have a history of allergies to biological agents or any drug components; those who

A subject who was diagnosed with severe or critical COVID-19 (as per the Diagnosis and Treatment Protocol for Novel Coronavirus Pneumonia (Trial, 8th edition), China, or Clinical Management of COVID-19, WHO);

Abnormal important organ function indicators, which meet the following conditions:

(1) Travel / residence history of communities with COVID-19 case reports within 14 days prior to the entry;

(2) Contact with SARS-CoV-2 infected persons (positive for nucleic acid detection) within 14 days prior to the entry;

Travel history / residence history of communities with case reports within 14 days;

History of contact with a SARS-CoV-2 infection (positive in nucleic acid test) within 14 days;

Have contacted patients with fever or respiratory symptoms from communities with case reports within 14 days;

Laboratory abnormalities of clinical significance, including white blood cell count, lymphocyte count, neutrophils, platelets, hemoglobin, alanine aminotransferase (ALT), aspartate aminotransferase (AST), total bilirubin, fasting blood glucose, and creatinine. Only apply to the sentinel participants in each cohort (the first 30 subjects);

Exclusion Criteria:

Family history of seizure, epilepsy, brain or mental disease

Subject allergic to any component of the investigational vaccine, or a more severe allergic reaction

1. have the medical history or family history of convulsion, epilepsy, encephalopathy and psychosis.

2. be allergic to any component of the research vaccines, or used to have a history of hypersensitivity or serious reactions to vaccination.

3. women with positive urine pregnancy test, pregnant or breast-feeding, or have a pregnancy plan

have the medical history or family history of convulsion, epilepsy, encephalopathy and psychosis.

be allergic to any component of the research vaccines, or used to have a history of hypersensitivity or serious reactions to vaccination.

women with positive urine pregnancy test, pregnant or breast-feeding, or have a pregnancy plan

1. Subjects receiving a surgery within 8 weeks prior to screening, or who are planned to receive a surgery prior to completion of the trial, which may cause unacceptable risks to the subjects as confirmed by investigators;

2. Use of the following drugs or therapies prior to administration of the investigational drug:

Those who have had surgery within 8 weeks prior to screening or who have surgery planned during the trial, and the investigator believes that such surgery may bring potential risks to the subjects.

Prior to administration of the investigational drug, use of the following drugs or therapies: i. any prescription drug within 28 days. ii. any over-the-counter drug, including health care products within

1. SARS-CoV-2 infection confirmed by positive reverse transcription-polymerase-chain-reaction (RT-PCR) assay.

2. A history of infection with SARS-CoV-2, or a history of contacting with cases of confirmed or suspected SARS-CoV-2 infection.

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RT-qPCR detects active SARS-Cov-2 infection;
a history of COVID-19, SARS, MERS infection (self-report, on-site inquiry);
a high-risk factor for COVID-19 infection:
a history of close contact with people infected with the new coronavirus, a history of living in high-

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Deviation in Masking (1 if Yes, 0 if not specified in one or more of data sources)	Masking	Deviation in Primary outcome (1 if Yes, 0 if not specified in one or more of data sources)
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Primary Outcome

time to clinical improvement, defined as the time from randomization to an improvement of two points (from the status at randomization) on a seven-category ordinal scale or live discharge from the hospital, whichever came first.

Clinical improvement time of 28 days after randomization

The 7-point scale

7 points: death

6 points: admission to ECMO and / or mechanical ventilation

5 points: Hospitalized for non-invasive ventilation and / or high-flow oxygen

the difference in the interval from baseline (initiation of antiviral treatment) to SARS-CoV-2 nucleic acid negativity by nasopharyngeal swab among the three antiviral treatment groups, with each of these two tests at least 24 h apart.

The time to 2019-nCoV RNA negativity in patients

the change in the disease severity category of COVID-19 after treatment. The severity of COVID-19 was assessed based on the Six-Point Clinical Status Scale for COVID-19 (COVID-19 severity scale) (Box 1). The Six-Point Clinical Status Scale for COVID-19 was defined according to NHC-NATCM-China

Critically ill patients (%)

the SARS-CoV-2 clearance rates in COVID-19 patients assessed on day six of antiviral treatment.

novel coronavirus nucleic acid clearance rate

the rate of achieving clinical symptom recovery on day 14 of treatment. The standard of clinical symptom recovery was the disappearance of the main symptoms, such as fever, cough, and fatigue.

Clinical symptom recovery rate

the time to clinical improvement, defined as the time from enrolment to an improvement of two points on a seven-category ordinal scale or live discharge from the hospital, whichever came first.

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disease remission
time to clinical improvement
overall rates of clinical improvement assessed at days 7, 14, and 28
time to chest CT improvement
time to virus nucleic acid negative-conversion

the total time of intubation, defined as the sum of the time spent from the beginning of bronchoscopy until proper tracheal tube placement was confirmed.

the lowest SpO2 during intubation

time to clinical improvement within a 28-day period. Clinical improvement was defined as patient discharge or a reduction of 2 points on a 6-point disease severity scale

the number of days between randomised grouping and clinical improvement

the rate of nucleic acid negativity conversion of SARS-CoV-2 and the negativity conversion time.

time and rate of temperature return to normal
time and rate of improvement of respiratory symptoms and signs (lung rhones, cough, sputum, sore throat, etc.)
time and rate of improvement of diarrhea, myalgia, fatigue and other symptoms
time and rate of pulmonary imaging improvement

whether patients had negative conversion of SARS-CoV-2 by 28 days and whether patients with severe covid-19 had clinical improvement by 28 days.

Viral nucleic acid test

primary safety data (infusional and allergic reactions, secondary infection and life-threatening adverse events) and the primary efficacy data (the level of the cytokines variation, the level of C-reactive protein in plasma and the oxygen saturation).

the median time of hospital stay

Improved respiratory system function (blood oxygen saturation) recovery time

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the time to clinical improvement, ie, the duration fromrandomization to the improvement of at least 1 point ona 7-category ordinal scale (adopted from a scale for hospitalized patients with severe influenza) or discharge from hospital, whichever occurred first

Clinical symptoms

Blood routine

the viral load of 2019-nCOV of throat swab

TBNK cell subsets

TH1/TH2 Cytokine

the time interval from admission to viral RNA negative conversion, patients were admitted to hospital on the day or the following day (<24 hours) after the diagnosis of the first positive RNA test

viral negative-transforming time

30-day cause-specific mortality

30-day cause-adverse events

30-day all-cause mortality

co-infections

Not specified

Complete Blood Count

CRP

blood coagulation

D-dimer

Virological examination of pharyngeal swab

the time from Leflunomide initiation to clinical improvement meeting the discharge criteria (set by the National Health Commission of China)

duration of viral shedding, which was defined as the time from randomization to the first negative nucleic acid test of five consecutive RT-PCR results.

The days from positive to negative for viral nucleic acid testing

HAMD17 and HAMA

the score of Hamilton depression scale

the score of Hamilton anxiety scale

the score of Self-Rating Depression Scale

the score of Self-Rating Anxiety Scale

the level of peripheral blood lymphocyte and IL-6 on the 1st, 7th, 14th day of the enrollment,

The percentage of patients who convert to moderate one
The rate of shock
Endotracheal intubation ratio
Time spent on the ventilator
mortality

the cumulative lung lesion remission rate (lung CT examination indicated absorption of lung inflammation).

Clinical cure rate

Phase 1:
Safety endpoints:
z The primary safety endpoint is the incidence of adverse reactions within 7 days after each injection:
primary safety outcome
The 7-day adverse reactions
primary humoral immunogenicity outcomes
neutralizing antibody titres and specific IgG binding antibody titres measured on

Incidence of adverse reactions/events

the occurrence of adverse reactions within 7 days after the first and second vaccinations.

the occurrence of adverse reactions within 7 days after each vaccination

Incidence of adverse reactions/events

the incidence of solicited local reactions at the injection site or systemic adverse reactions in the 14 d after the prime or boost dose, and adverse events after the full immunization in the 28 d after receiving the boost dose. Any clinical laboratory abnormalities from baseline to 24 h or 7 d after the prime or boost.
Occurrence of solicited local reactions in the subjects (e.g., vaccination sites: pain/tenderness, erythema/redness, induration/swelling) during the 14-days after each dose of BNT162b1 or placebo.
Occurrence of solicited systematic reactions (e.g., nausea, vomiting, diarrhea, headache, fatigue, myalgia, arthralgia, chills, loss of appetite, malaise, and
Phase I
Incidence of adverse events within 28 d after each dose of vaccine.

Phase II

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Incidence of adverse reactions/events

The seropositive rates and level of SARS-CoV-2 neutralizing antibody
The seropositive rates of SARS-CoV-2 IgG antibody (tested by ELISA)

The primary endpoints for immunogenicity were neutralization antibody seroconversion and titre, and RBD-IgG seroconversion 28 days after the third dose.

The seropositive rates and level of SARS-CoV-2 neutralizing antibody
The seropositive rates of SARS-CoV-2 IgG antibody (tested by ELISA)

depression, anxiety and stress levels using the Chinese Version of Depression Anxiety and Stress Scale-21 (DASS-21)

Psychological health level of the patient with COVID-19, assessed using the Chinese Version of Depression Anxiety and Stress Scale-21 (DASS-21) for all participants at baseline and post- intervention (1 day later)

the incidents of ARDS development;

The incidents of acute respiratory distress syndrome (ARDS) development

viral clearance rate at day 7 after randomization

The virological clearance rate of throat swabs, sputum, or lower respiratory tract secretions at day 7

the rate of positive-to-negative conversion of SARS-CoV-2 nucleic acid from the initiation of treatment to day 21, with the enrollment day as the first day of treatment.

The rate of virus inhibition

time to clinical improvement within 28 days after randomisation.

Time to Clinical Improvement (TTCI)

主要研究终点为第7天咽拭子、痰液或下呼吸道分泌物等样本病毒学清除或2周内患者发生死亡

The primary endpoint was negative conversion rate of SARS-CoV-2 nucleic acid in respiratory pharyngeal swab on days 7 after randomization

The virological clearance rate of throat swabs, sputum, or lower respiratory tract secretions at day 3

The virological clearance rate of throat swabs, sputum, or lower respiratory tract secretions at day 5

The virological clearance rate of throat swabs, sputum, or lower respiratory tract

invasive mechanical ventilation (IMV)-free days in 28 days (IMVFD28).

Ventilation-free days

the incidence of clinical deterioration 14 days after randomization.

the incidence of treatment failure in 14 days

The primary end points were the time to clinical recovery and the deterioration rate after initiation of medications.

Time to clinical recovery after treatment

Rate of aggravation

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change in the total lesion proportion (%) of the whole lung volume from baseline to day 28, as measured by chest CT. It was defined as (total lesion proportion of the whole lung volume at day 28–total lesion proportion of the whole lung volume at baseline)/total lesion proportion of the whole lung volume at baseline

change in the total lesion proportion (%) of the whole lung volume from baseline to month 1

Change in lesion proportion (%) of full lung volume from baseline to day 28.

The primary endpoint for safety evaluation was the incidence of adverse reactions within 14 days after the injection. The primary endpoints for immunogenicity were the geometric mean titres (GMTs) of RBD-specific ELISA antibody responses and neutralising antibody responses against live virus or

Occurrence of adverse reactions
Anti SARS-CoV-2 S IgG antibody response(ELISA)
Neutralizing antibody response to SARS-CoV-2

Phase I
Incidence of adverse reactions occur from the beginning of the vaccination to 28 days after the whole schedule vaccination
Phase II

Safety indexes of adverse reactions
Immunogenicity indexes of neutralizing-antibody seroconversion rates

The primary safety endpoint was any vaccine-related adverse event (adverse reaction) within 28 days after the administration of each dose of vaccine or placebo. The primary immunogenicity endpoint was the seroconversion rate of neutralising antibodies to live SARS-CoV-2 at day 28 after the second dose.

Safety index-incidence of adverse reactions
Immunogenicity index-seroconversion rates of neutralizing antibody

Phase 2 trial among adults aged 18-59 years old
Primary endpoint
- Positive conversion rate of serum neutralizing antibody on Day 14 (Cohort 1)/Day 28 (Cohort 2) after two doses of test vaccine;

Safety indexes of adverse reactions
Immunogenicity indexes of neutralizing-antibody seroconversion rates

Safety index-incidence of adverse reactions
Immunogenicity index-seroconversion rates of neutralizing antibody

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the seroconversion rates of the anti–SARS-CoV-2 neutralizing antibody and ELISA immunoglobulin G (IgG) antibody at day 14 (0, 14 procedure) and day 28 (0, 28 procedure), respectively, after the boost immunization.

The primary end point was the total rate of adverse reactions from 0 to 28 days post-immunization.

Adverse reactions/events rate

Adverse reactions/events rate

Seroconversion rate of Neutralizing antibodies against SARS-CoV-2 Phase IIa (Day 0, 14 schedule)

Seroconversion rate of Neutralizing antibodies against SARS-CoV-2 Phase IIa

Not specified

Correlation of adverse events with the investigational product

For the phase 1 trial, the primary outcome was the safety of the COVID-19 vaccine. For the phase 2 trial, primary outcomes were safety and immunogenicity.

Safety outcomes were the occurrence of adverse events between the first

The number of adverse events after intramuscular injection

Neutralizing antibody positive conversion rate

Phase 1 trial: the incidence of adverse reactions within 7 days after each dose vaccination

Phase 2 trial: the GMT of RBD-specific ELISA antibody responses at 30 days after the last dose vaccination and the incidence of adverse reactions within 7

Occurrence of adverse reactions (AR) within 7 days after each dose of the recombinant SARS-CoV-2 vaccine (Sf9 cell) or placebo.

Geometric mean (GMT) of specific antibody

The incidence of adverse reaction (AR)

the percentage of subjects with viral negative by Day 14

the time from randomization to clinical improvement

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Time to viral negativity by RT-PCR
Time to clinical improvement

symptom improvement rate and symptom disappearance rate after 14 days of treatment;

clinical symptom improvement and disappearance rates after 14 days of treatment

the negative conversion ratio
the proportion of general patients with advanced severe disease
confirmed rate of suspected cases

the rate of worsening before clinical endpoint occurrence

Rate of conversion to severe or critical illness

clinical improvement

14 day outcome of the subjects, including: recovery, improvement, turning critical, death.
lung CT

SOFA Score

SOFA

The primary outcome in the study of moderate COVID-19 was time to clinical recovery.
The primary outcome in the study of severe COVID-19 was time to clinical improvement.

TTCI (Time to Clinical Improvement)

Time to Clinical Recovery, TTCR

modified Medical Research Council (mMRC) dyspnea scale

lung function
ADL
6min walk

The time required for a negative SARS-CoV-2 RNA result or the rate of negative results on day 10.

COVID-19 nucleic acid detection time from positive to negative (respiratory secretion) or (3,5,7,10 days from positive to negative rate)
Lung CT observation of inflammation absorption
Proportion of cases with progressive disease

the conversion time of the SARS-CoV-2 viral assay

The time to 2019-nCoV RNA negativity in patients

the difference between carbon dioxide (CO2) concentration in the examination room at baseline and the peak value during examination
adenosine triphosphate (ATP) fluorescence value

During the operation, the patient's volume of local exhaled air from the mouth and nose, the patient's heart rate, respiratory rate and blood oxygen saturation

退热时间

fever clearance time
Effective rate of TCM symptoms

PaO2/FiO2
ROX index (ROX=SpO2/(FiO2*RR))

PO2/FiO2
ROX INDEX

the rate of symptom recovery after 14 days of treatment

The time for the positive nucleic acid of the novel coronavirus to turn negative
Incidence of deterioration
The defervescence time
Chest CT

6 min walking distance (6MWD) in metres.

Total distances of 6 minutes walk test

improvements in 6MWD and findings on chest computed tomography (CT) images.

St George's respiratory questionnaire score
Fatigue assessment scale
Six minute walking experiment

incidence of adverse events or adverse reactions within 60 min, and at days 7, 14, and 28 after each vaccine dose.

Specific IgG antibody
Neutralizing antibody
Cellular immunity

PTSS of COVID-19 patients accessed before and after intervention

Photo - C grade
SAS
SDS
PSQI

The primary immunogenic endpoints were the seroconversion rates [geometric mean titer (GMT) ≥ 16] and GMT of neutralizing antibody to live SARS-CoV-2 at day 28 after the last dose.
The primary endpoint for safety was the occurrence of adverse reactions within

Serum neutralizing antibodies, IgG antibodies, and IgM antibodies of SARS-CoV-2 28 days after the last dose of vaccination
Serious adverse events (SAE) during the entire study period (6 months after the full vaccination

Serum neutralizing antibodies, IgG antibodies, and IgM antibodies of SARS-CoV-2 28 days after the last dose of vaccination

The primary endpoint was the rate of positive nucleic acid tests performed on nasal and pharyngeal swabs during the quarantine medical observation period.

Nucleic acid test positive rate of nasal and throat swabs during the isolation medical observation period

The primary immunogenicity outcomes were the seroconversion rate, geometric mean titer (GMT) of the RBDbinding antibody and SARS-CoV-2 neutralizing antibody.
1. Positive conversion rate of serum anti-SARS-CoV-2 RBD protein antibody, and its geometric mean titer (GMT) and geometric mean fold increase (GMI)
2. Positive conversion rate of serum anti-SARS-CoV-2 neutralizing antibody, and its GMI
The safety outcomes were the frequencies and percentages of AEs within seven days after the vaccination, AEs between the first vaccination to 30 days after the final vaccination, any abnormal changes in laboratory measures at seven days post-vaccination, and SAEs during the whole follow-up period. AEs,

Safety

time to achieve a consecutive twice (at intervals of more than 24 h) negative RT-PCR result for SARS-CoV-2 RNA in nasopharyngeal swab and sputum sample.

Viral nucleic acid test negative conversion rate

DLT and MTD of SCTA01

Dose-limiting toxicity(DLT)
Maximal Tolerable Dose(MTD)

the safety and tolerability of MW33	
Adverse Event Serious Adverse Event	
SARS-CoV-2 viral load	
Primary Endpoint - Incidence rate of adverse reactions occured from the beginning of the vaccination to 28 days after the second dose vaccination	
Primary Endpoint	
Safety index-incidence of adverse reactions Immunogenicity index-seroconversion rates of neutralizing antibody	
the incidence of ARs within 14 days after each vaccination and the GMTs of RBD-specific ELISA antibodies and pseudovirus neutralizing antibodies on day 28 after boost vaccination	
Safety indexes of adverse reactions Immunogencity indexes of GMT Immunogencity indexes of neutralizing antibody	
The occurrence of adverse reactions within 28 d after boosting was evaluated as the primary endpoint for safety	
The GMT of neutralizing antibodies against live SARS-CoV-2 virus at 14 d after boosting was the primary immunogenic endpoint.	
Incidence of adverse reactions within 28 days after the booster dose.	
GMT of neutralizing antibodies against live SARS-CoV-2 virus on day 14 after the booster dose.	
Not specified	
Treatment-emergent Changes in Body Temperature Treatment-emergent Changes in Respiratory Rate Treatment-emergent Changes in Pulse Rate Treatment-emergent Changes in Blood Pressure	
Not specified	

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Immunogenicity of Third Dose Vaccine Booster Following Two Doses of Inactivated Vaccines

Reactogenicity of Third Dose Vaccine Booster Following Two Doses of Inactivated Vaccines

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Secondary Outcome

Other clinical outcomes included clinical status as assessed with the seven-category ordinal scale on days 7 and 14, mortality at day 28, the duration of mechanical ventilation, the duration of hospitalization in survivors, and the time to clinical improvement (TTCI)

Proportion of patients in each category of the 7-point scale on days 7, 14, and 28 after randomization;

mortality on day 28

Duration of mechanical ventilation(days)

the differences among the three groups in the proportion of patients with SARS-CoV-2 nucleic acid negativity at day 14,

the mortality rate at day 28,

mortality on day 28

The rate of negative 2019-nCoV RNA results at day 14

the rate of aggravation

the rate of adverse events

The rate of discontinuations due to adverse events

the overall survival through last day of treatment, the proportion of patients without improvement (scored 3–5 on the COVID-19 severity scale), the change in serum procalcitonin level after treatment, and the prevalence of antibiotic use during treatment

Oxygenation index

PCT

Antibiotic use

the median time to SARS-CoV-2 clearance.

time till the SARS-CoV-2 clearance

the proportion of patients who achieved clinical symptom recovery on days 7 and 10,

the time for clinical recovery of the patients,

the time for nucleic acid turning negative (defined as two consecutive

clinical symptom disappearance time

Time and rate of virus turning negative

Length of hospital stay

Antipyretic time

Severe conversion rate

the time to radiological improvement defined as the time from enrolment to radiological improvement on chest CT scans,

the time to virus nucleic acid negative conversion defined as the time from enrolment to two consecutive negative tests for SARS-CoV-2 via RT-PCR

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time to discharge
mortality
deterioration rate
adverse events

the lowest SpO2 during intubation, incidence of mask ventilation for SpO2 <90%, PaO2/FIO2 before intubation, incidence of SpO2 <80% during intubation, incidence of minimum SpO2 >95% during intubation, and 7 day mortality

the incidence of hypoxia
the times of intubation
Jaw thrust maneuver
Length of stay in ICU
Death

(1) 28-day mortality, including analysis of time from randomization to death; (2) duration of hospitalization, including analyses of time from randomization to discharge, time from admission to discharge, and 28-day discharge rates; and

28-day mortality
Hospitalization time
ICU hospitalization
Invasive mechanical ventilation during infection
ECMO duration during infection

Secondary outcomes were the improvement rate of chest CT images, the time required for the body temperature returning to normal and the improvement in respiratory symptoms and signs.

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Probability of negative conversion at day 4, 7, 10, 14, or 21 was specified as a secondary outcome in the protocol, but this does not appear on the trial registration list. Other prespecified secondary outcomes not listed in the trial registration but included in the protocol

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The secondary efficacy outcomes mainly included the total lymphocyte count and subpopulations, the chest CT, the respiratory rate, and the patient symptoms (especially the fever and shortness of breath). In addition, the therapeutic measures (i.e. antiviral medicine and respiratory support) and

the levels of serum C-reactive protein (CRP)

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Secondary outcomes included lymphocyte cell counts on day 5 posttreatment, mortality at day 21, the proportions of patients progressing to critical conditions (eg, acute respiratory distress syndrome, sepsis, or septic shock), and viral loads (reflected by the Ct values as measured with real-time

#BLANK

Secondary outcomes included chest computed tomography (CT) improvement time, symptom improvement/disappearance time, viral shedding duration, length of hospital stay, mortality, and complications during treatment. Patients were followed up to 28 days after discharge. Other

#BLANK

Not specified

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Other clinical outcomes included length of hospitalization, duration of viral shedding, and safety outcomes.

Other clinical outcomes included clinical status, i.e. progressive rate to severe illness, syndromes, peripheral blood cells and biochemical parameters, C-reactive protein and inflammatory cytokines, length of hospital stay, etc. Safety outcomes included adverse events that occurred during

Rate of progress to severe illness
Duration of fever
C reactive protein concentration in peripheral blood
Absolute value and percentage of lymphocytes in peripheral blood

Secondary end points for evaluating the efficacy of cCBT were SDS to assess self-rated depressive symptoms, SAS to assess anxiety symptoms, and AIS to assess insomnia symptoms.

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(1) 28-day follow-up mortality, the rate of mechanical ventilation, septic shock and conversion to the moderate type during 14 days of treatment.
(2) Peripheral blood leukocytes, C-reactive protein (CRP), IL-8 and TNF-α on the 1st, 7th, 14th day of the enrollment. Sample collection was carried out

CRP
IL-6
TNF-α
Time of improvement of clinical symptoms (fever, cough)
Adverse drug reactions

the improvement of clinical symptoms (cough, diarrhea, dyspnea, fever, myalgia) before and after treatment;
the changes of blood routine test and IL-6;
Viral nucleic acid test negative conversion rate and days from positive to negative
Duration of fever
Lung imaging improvement time
Mortality rate because of Corona Virus Disease 2019

Phase 1:
The secondary safety endpoints include:
z Incidence of any adverse reactions/events within 30 minutes after each injection;

Secondary immunogenicity outcomes
neutralizing antibody titres and specific IgG binding antibody titres

Secondary safety outcome
Incidence of abnormal indicators of liver and kidney function, blood routine and urine routine
Rate of adverse reaction and adverse event
Incidence of Serious Adverse Events (SAE)

Any abnormal changes in laboratory measures at day 4 post inoculations, and adverse events within 28 days after the first and the second vaccinations across the treatment groups were analysed as secondary safety endpoints.
The secondary humoral immunogenicity outcomes were measured using an adverse reactions within 30 days after whole vaccination procedure across all study groups were analysed as secondary safety endpoints.
The secondary outcomes for humoral immunogenicity were measured with infectious SARS-CoV-2 neutralising assay (on days 0, 28, 56, and 84) and
Incidence of abnormal indicators of liver and kidney function, blood routine and urine routine
Incidence of adverse reactions/events
Incidence of Serious Adverse Events (SAE)
Four-fold growth rate and antibody level (GMT, GMI) of serum antibody

The secondary endpoints for immunogenicity were GMT, seroconversion rate and fold increase of virus-neutralizing antibody, or ELISA IgG antibodies binding to S1 or RBD measured 7 d and 21 d after both the prime dose and the boost dose.
The proportion of subjects experiencing serious adverse events (SAEs), occurring up to Day 21 after prime vaccination and Day 28 after boost vaccination, up to Month 3, 6 and 12.
The proportion of subjects experiencing AE associated with BNT162b1, occurring up to Month 3, 6 and 12

Phase I
Incidence of serious adverse events from the first dose to 12 months after full-course immunization.
Incidence of abnormal blood routine, blood biochemistry, coagulation

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Serious Adverse Events(SAE)
Abnormal indicators of blood routine, blood biochemistry, urine routine and coagulation function
The seropositive rates and level of SARS-CoV-2 neutralizing antibody and IgG antibody(tested by ELISA)

Incidence of adverse reactions/events
Serious Adverse Events (SAE)
Level of SARS-CoV-2 IgG antibody (tested by ELISA)
The seropositive rates and level of SARS-CoV-2 neutralizing antibody and IgG antibody (tested by ELISA)

proportion of participants experiencing adverse reactions/events within 28 days following each vaccination,
occurrence of SAE from the first dose through 12 months post the third dose,
titre of RBD-IgG 28 days after the third dose,

Incidence of adverse reactions/events
Serious Adverse Events (SAE)
Level of SARS-CoV-2 IgG antibody (tested by ELISA)
The seropositive rates and level of SARS-CoV-2 neutralizing antibody and

Not specified

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the time to fever resolution
the recovery of lung injury

The time to fever resolution rate
Time to recovery of lung injury

viral clearance at day 3 and day 5,
the critical illness rate of subjects during the 14 days after randomization,
the mortality rate of subjects at day 14, and
the number of participants with treatment-related adverse events

The virological clearance rate of throat swabs, sputum, or lower respiratory tract secretions at day 3
The virological clearance rate of throat swabs, sputum, or lower respiratory tract secretions at day 5
Number of participants with treatment-related adverse events as assessed

1) the rate of positive-to-negative conversion of SARS-CoV-2 nucleic acid at day 14;
2) the rate of antipyresis (defined as axillary temperature ≤ 37.3°C for more than 72 hours) from the first day of treatment;

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The disease prorogation-temperature
The disease prorogation-respiratory function 1
The disease prorogation-respiratory function 2
The disease prorogation-respiratory function 3

the proportions of patients in each category of the six-point scale at day 7, 14, and 28 after randomisation;
all-cause mortality at day 28;
frequency of invasive mechanical ventilation;

Clinical status
Time to Hospital Discharge OR NEWS2 (National Early Warning Score 2) of ≤ 2 maintained for 24 hours.
All cause mortality
Duration (days) of mechanical ventilation

次要研究终点为2周内发生严重药物不良反应或受试者病情转为重型、危重型

Number of participants with treatment-related adverse events as assessed by CTCAE v5.0
The critical illness rate of subjects at weeks 2

28-day mortality,
organ functions and inflammatory parameters, including white blood cell counts, neutrophil counts, lymphocyte counts, procalcitonin, interleukin-6 (IL-6), and C-reactive protein (CRP).

28-days mortality
ICU length of stay
Demand for first aid measuments
Vasopressor days
Respiratory indexes

the incidence of clinical cure 14 days after randomization,
the incidence of intensive care unit (ICU) admission,
in-hospital mortality,
the time from randomization to clinical cure

clinical cure incidence in 14 days
the duration of virus change to negative
mortality at day 30
ICU admission rate in 30 days

The secondary outcomes assessed the virologic clearance during the study period (SARS-CoV-2-negative conversion within 20 days, rate of SARS-CoV-2-negative conversion), clinical follow-up (clinical remission rate, time to fever remission, rate of significant improvement in chest CT, percentage of

Clinical remission rate
Dynamic changes of oxygenation index
Time to cure
rate to cure
Time to defervescence

The secondary imaging outcomes were a change in the total lesion proportion (%) of the whole lung volume from baseline to day 10, a change in solid component and ground-glass lesion proportion from baseline to day 10, 28, and change in lung densitometry at day 10, 28. Clinical outcomes within

change in the total or solid component lesion proportion (%) of the whole lung volume from baseline to followup

Change in lesion proportion (%) of full lung volume from baseline to day 10 and 90

Change in consolidation lesion proportion (%) of full lung volume from baseline to day 10, 28 and 90.

The secondary endpoints for immunogenicity were RBD-specific ELISA antibody responses at day 14 and month 6 (6 month data not yet available), and specific T-cell responses at day 28 post vaccination. Seroconversion of the humoral immune responses was also a secondary endpoint, and was

Occurrence of adverse events

Occurrence of serious adverse reaction

Anti SARS-CoV-2 S IgG antibody response(ELISA)

Neutralizing antibody response to SARS-CoV-2

Neutralizing antibody response to Ad5-vector

Phase I

Incidence of adverse reactions within 7 days after each dose of vaccination ;

Incidence of abnormal laboratory index (blood routine test, blood chemistry

Safety indexes of adverse reactions

Safety indexes of Incidence of abnormal laboratory index

Safety indexes of serious adverse events (SAEs)

Immunogenicity indexes of the seropositive rate, GMT, and GMI of neutralizing antibodies

Secondary endpoints were severe adverse events and geometric mean titre (GMT) of neutralising antibodies to live SARS-CoV-2, as well as seropositive rates and geometric mean increase.

Safety index-incidence rate of adverse reactions

Safety index-incidence rate of serious adverse events in phase Ⅰ

Safety index-incidence rate of serious adverse events in phase Ⅱ

Immunogenicity index-seropositive rate, GMT, and GMI of neutralizing antibodies

Secondary endpoint

- Positive rate, GMT and GMI of serum neutralizing antibody on Day 28 after two doses of test vaccine;

- Positive conversion rate, positive rate, GMT and GMI of serum neutralizing

Safety indexes of adverse reactions

Safety indexes of Incidence of abnormal laboratory index

Safety indexes of serious adverse events (SAEs)

Immunogenicity indexes of the seropositive rate, GMT, and GMI of

Safety index-incidence rate of adverse reactions

Safety index-incidence rate of serious adverse events in phase Ⅰ

Safety index-incidence rate of serious adverse events in phase Ⅱ

Immunogenicity index-seropositive rate, GMT, and GMI of neutralizing

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(1) the total AE rate from 0 to 7 days following each inoculation and from 0 to 28 days after the boost immunization,
(2) the geometric mean titer (GMT) profiles of the anti-SARS-CoV-2 neutralizing antibody and ELISA IgG antibody at days 14 and 28 (0, 14

The secondary end points were serological evidence of the immunogenicity of the vaccine.

Serious adverse events
Level of Neutralizing antibodies against SARS-CoV-2 Phase Ia (Day 0, 14 schedule)
Level of Neutralizing antibodies against SARS-CoV-2 Phase IIa (Day 0, 14 schedule)

Not specified

Primary pharmacokinetic variables
Primary pharmacokinetic variables
Primary pharmacokinetic variables
Primary pharmacokinetic variables

For the phase 1 trial, the secondary outcome was immunogenicity. Outcomes for immunogenicity were the seroconversion rate and magnitude, in geometric mean titres (GMTs), of the RBD-binding antibody, SARS-CoV-2 neutralising antibody, and T-cell cytokine production (primary for phase II).

Immunogenic end point

Neutralizing antibody GMT, positive rate
Neutralizing antibody GMT, positive conversion rate/positive rate
Neutralizing antibody GMT
Neutralizing antibody GMT
Neutralizing antibody GMT, positive rate

The secondary endpoints in phase 1 trial for safety were the incidence of adverse events and serious adverse events from the first dose to 28 days after the last dose, and abnormal changes in laboratory measures at 3 days after each dose vaccination.
Occurrence of adverse events (AE) within 7 days after each dose of the recombinant SARS-CoV-2 vaccine (Sf9 cell) or placebo.
Occurrence of AE up to Day 28 after prime and boost vaccination.
The proportion of SAEs up to Day 28 after prime and boost vaccination.
The incidence of adverse events (AE)
The incidence of severe adverse events (SAE)
The incidence of serious adverse events
Geometric mean (GMT) of anti-SARS-CoV-2-specific neutralizing antibodies
the percentage of subjects with viral negative by Day 7
the incidence of mechanical ventilation by Day 14
ICU admission by Day 14
all-cause mortality by Day 14

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Incidence of mechanical ventilation by day14
Incidence of ICU admission by day14
Time to treatment failure
All-cause mortality by day14, day28

the disease aggravation rate after 14 days of treatment

the proportion of patients who progressed to severe status despite the same duration of treatment

Traditional Chinese medical syndrome scale
Body temperature

discharge with cure
composite symptom outcome

Rate of disisolation
TCM symptom
Body temperature
Blood routine test

mortality
rate of advancing to critical category
receiving an invasive ventilator in the severe category
vital signs
oxygenation index
Routine Blood
inflammatory biomarkers

all-cause mortality up to 30 days

cure rate
All-cause mortality
Severity rate

The secondary outcomes in the study of moderate COVID-19 included:
(1) 28-day mortality,
(2) time to virological clearance,
(3) frequency of dyspnea progression,
Proportion of subjects with 6-grades rating form at fixed time points (days 7, 14, 21, and 28 after randomization)
All-cause mortality of 28-days
Duration of mechanical ventilation (days) of 28-days

All-cause mortality of 28-days
Transition time of 2019-nCoV nucleic acid detection
Frequency of respiratory progression：Defined as SPO2≤ 94% on room air or PaO2/FiO2 <300mmHg and requirement for supplemental oxygen or more
the modified Borg dyspnea scale (MBS)
fatigue Scale-14 (FS-14)
patient health questionnaire-9 scale (PHQ-9)
duration of respiratory symptoms
PHQ-9
Length of hospital stay
Respiratory symptoms
immune cells

The improvement rate and time of cardinal symptoms (axillary temperature, transcutaneous oxygen saturation, and respiratory frequency recovery)

#BLANK

1) the change in the 7-point scale;
2) the rate of critical aggravation;
3) blood routine test outcomes;
4) blood biochemical test outcomes;
The 7-point scale
The rate of critcal aggravation
Blood routine test outcomes
Blood biochemical test outcomes

adverse events
COVID-19
examination time

New coronavirus infection before and after treatment, and new coronavirus infection among relevant medical staff (including chest CT, viral nucleic acid detection, clinical diagnosis, etc.)

入院第 1 天及研究第 7 天，记录单项症状消失情况

body tempture
lung CT scan
clearance time of COVID-19
Oxygen saturation

the white blood cell (WBC) count
peripheral blood mononuclear lymphocyte (PBML) count
respiratory support step-down time (RSST)

WBC
PBML
Time of Support time

the time to recovery of individual symptoms,
TCM syndrome scores,
the time of conversion of SARS-CoV-2 RNA assay,
hospitalization days,
Haematological
C-reactive protein
Blood gas analysis
Urine test
squat time in seconds;
pulmonary function assessed by spirometry;
HRQOL measured with Short Form Health Survey-12 (SF-12)
mMRC-dyspnoea
pulmonary function
mMRC dyspnea scale
strength of lower limber muscles
ICF-COVID simple evaluation form
Chinese medicine syndrome scores,
St. George's Respiratory Questionnaire (SGRQ) score,
Borg-Dyspnea Scale scores,
Fatigue Assessment Inventory (FAI) scores

#BLANK

The secondary endpoint for safety was abnormal changes in laboratory tests at days 1, 4, 7, and 28 after each vaccine dose.

For immunogenicity, the secondary endpoint was titres of neutralising

#BLANK

depression, anxiety, and sleep quality

#BLANK

Secondary immunogenic endpoints were the positive rates (GMT ≥ 32, 64, 128, 256) 28 days after the whole course of vaccination, respectively. Adverse events within 28 days after the first and the second vaccinations across the three groups were analyzed as secondary safety endpoints.

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Serum neutralizing antibodies, IgG antibodies, and IgM antibodies of SARS-CoV-2 after enhancing immunity of vaccination
The specific immune cell level of SARS-CoV-2 in whole blood 28 days after the last dose of vaccination
The specific immune cell level of SARS-CoV-2 in whole blood 28 days after the last dose of vaccination
Serum neutralizing antibodies, IgG antibodies, and IgM antibodies of SARS-CoV-2 after enhancing immunity of vaccination
the time of the occurrence of positive nucleic acid tests of nasal and pharyngeal swabs during the quarantine medical observation period;
the proportion of asymptomatic people with positive nucleic acid tests;
the proportion of mild, medium, and severe patients with positive nucleic acid
Time of occurrence of positive nucleic acid tests in nasal and throat swabs during the isolation medical observation period
Proportion of asymptomatic infections among positive nucleic acid tests
Proportion of mild, normal, and severe patients with positive nucleic acid

Not specified

Safety outcomes
Immunogenicity outcomes

Not specified

Immunogenicity

the changes of blood routine and CRP (C-reactive protein);
the count and proportion of T lymphocyte subsets in peripheral blood and the changes of cytokines

Clinical cure rate

The secondary endpoints were the PK parameters, including the integral of the concentration-time curve from dosing to time point (AUC0–t), the integral of the concentration-time curve from dosing to infinity (AUC0–∞), half-life (t1/2), the time taken to reach the maximum concentration (Tmax),
AUC0-t
AUC0-∞
t1/2
Tmax

PK and assessment of ADAs to MW33

#BLANK

#BLANK

Secondary Endpoints

- Incidence rate of adverse reactions within 7 days after each dose vaccination;

- Incidence of abnormal laboratory index (blood routine test, blood chemistry

Safety index-incidence of serious adverse events

Immunogenicity index-seropositive rates of neutralizing antibody

Immunogenicity index-geometric mean titer (GMT) of neutralizing antibody

Immunogenicity index-geometric mean ratio (GMR) of neutralizing antibody

adverse events within 28 days after each vaccination,

serious adverse events reported up to 6 months,

any abnormal changes in laboratory measures at 4 days after each vaccination for sentinel participants,

Safety indexes of adverse events

Safety indexes of Hematological examination measures (Hemoglobin, WBC)

Safety indexes of Blood routine measures(ALT, AST)

Incidence of solicited adverse reactions within 14 d,

unsolicited adverse reactions within 28 d after boosting

serious adverse events reported during the 6-month study period

eceptor-binding domain (RBD)- and nucleocapsid (N)-specific

Incidence of solicited AE within 14 days after the booster dose

Incidence of unsolicited AE within 28 days after the booster dose.

Incidence of serious adverse events (SAE) till the 6 months after the booster dose.

Not specified

Assess Pharmacokinetic parameters of LY-CovMab

Assess Pharmacokinetic parameters of LY-CovMab

Assess Pharmacokinetic parameters of LY-CovMab

Assess Pharmacokinetic parameters of LY-CovMab

Not specified

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The seropositive rate of neutralizing antibodies
GMT of neutralizing antibodies
GMFR of neutralizing antibodies
severe adverse events and adverse events of special interest

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New primary outcome introduced in publication (1 = Yes, 0 = No)	Original primary outcome omitted in publication (1 = Yes, 0 = No)
1	1
0	1

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0	0
0	1
1	0
1	1
1	1

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1	1
0	1
1	0
0	0
0	1

peer review

1	1
0	1
0	0
0	1

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1 1

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1 0

1 1

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1	1
1	1
0	1

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1	1
0	1
1	1
0	0

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0	1
1	0
1	1
1	1
0	0

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1	0
0	0

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Secondary outcome in registry described as primary outcome in publication (1 = Yes, 0 = No)	Primary outcome in registry described as secondary outcome in publication (1 = Yes, 0 = No)
0	0
0	1

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1	1
0	1
0	0
0	0
0	0

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0	1
0	1
0	1
0	1
1	1

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0	0
1	0
1	0
1	0

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0 0

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0 0

0 0

For peer review only

0	0
0	1

0	1

For peer review only

0	0
0	0
0	0
0	1

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0	1
0	1
0	1
0	1
1	0

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0	0
1	0

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Deviation in time of Primary outcome Measurement(1 if Yes, 0 if not specified in one or more of data sources)
0

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For peer review only

For peer review only

For peer review only

0

1

1

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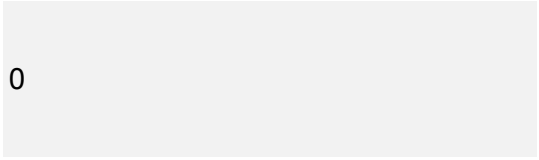
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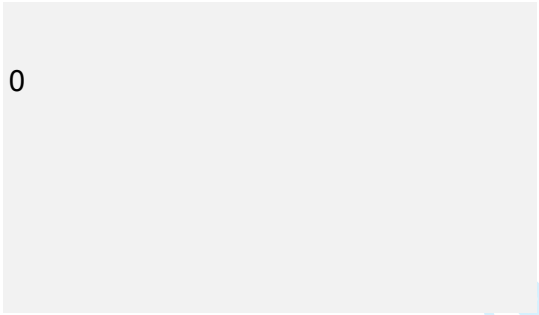
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0

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0

1

1

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For peer review only

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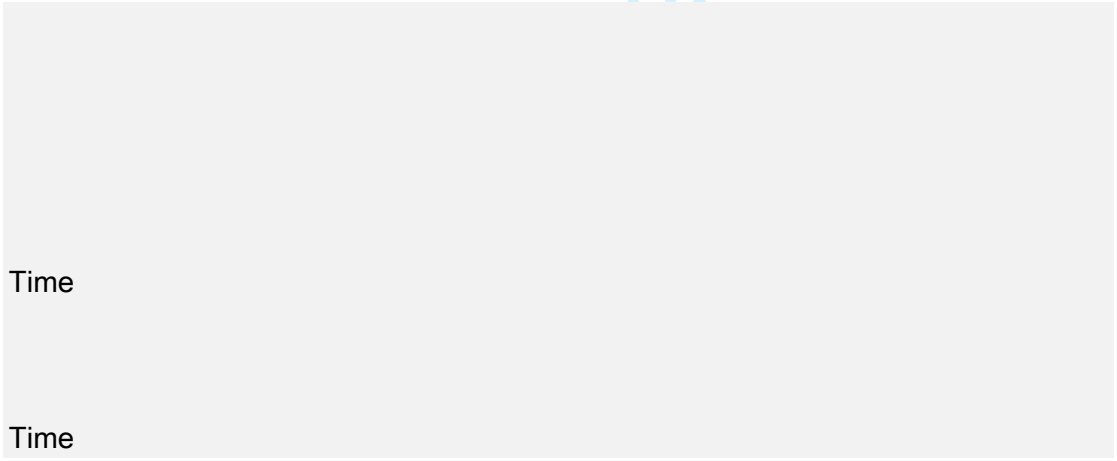
Time of Primary Outcome Measurement
Time
Time
Time
Time
day six of antiviral treatment
day 6 after treatment
on day 14 of treatment
Not Specified

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Time

Time

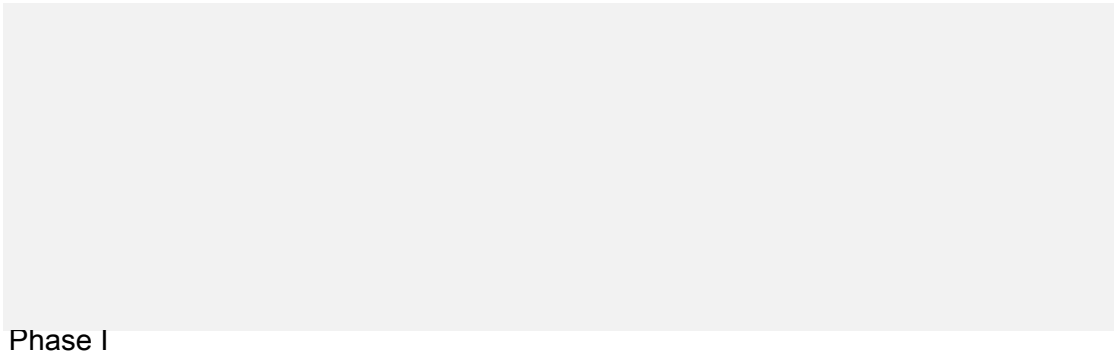
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Time
Time

within 7 days after the first and second vaccination

within 7 days after each vaccination

0-7 days after each dose of vaccination



Phase I
within 28 d after each dose of vaccine
Phase II
28 days after the whole immunization

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0-28 days after each dose of vaccination

Day 28 post full vaccination

28 days after the third dose

Day 28 post full vaccination
Day 28 post full vaccination

Not Specified

1 day later

28 days

Time Frame: 14 days

day 7 after randomization

at day 7

from the initiation of treatment to day 21

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Time Frame: Day 0, 2, 4, 7, 10, 14 and 21

Time

Time Frame: up to 28 days

Day7

Day3, Day5, Day7

14 days after randomization

in 14 days

within 14 days

Time Frame: within 14 days from the start of medication
Time Frame: within 14 days from the start of medication

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from baseline to day 28

month 1

Time Frame: Day 28

within 14 days after the injection
at day 28 post vaccination
at day 28 post vaccination

Time Frame: 0-14 days post vaccination
Time Frame: 28 days post vaccination
Time Frame: 28 days post vaccination

from the beginning of the vaccination to 28 days after the whole schedule vaccination
on the 14th (for the emergency immunization schedule) or 28th (for the routine immunization schedule) day after the whole schedule vaccination

Time Frame: From Day 0 to Day 28 after each dose
Time Frame: The 14th day /the 28th day after two doses of vaccination

Phase 2 trial among adults aged 18-59 years old

on Day 14 (Cohort 1)/Day 28 (Cohort 2) after two doses of test vaccine;
on Day 0-28 (Day 0-14 for the first dose for Cohort 1) after each dose;

Time Frame: From Day 0 to Day 28 after each dose
Time Frame: The 14th day /the 28th day after two doses of vaccination

Time Frame: Day 0-28 after each dose vaccination
Time Frame: 28 days after the second dose vaccination

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Phase I: within 7 days after each dose vaccination
Phase II: at 30 days after the last dose vaccination and within 7 days after each dose vaccination

Time Frame: 7 days after each dose.

Time Frame: 1 months after immunization in each study group
Time Frame: 0 to 7 days after vaccination in each study group

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Not Specified

Not Specified

7 days

Not Specified

28 days

Time (within 28 days after randomization)

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Time (within 28 days after randomization)

Time

Time

Not Specifed

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Not Specifed

6 weeks

Not Specifed

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Not Specified

Not Specified

Time

Time Frame: 5 months

84 days

Time Frame: 7 days
Time Frame: 12 weeks

from the beginning of the vaccination to 28 days after the second dose vaccination
28 days after the second dose vaccination.

Time Frame: Day 0-28 after each dose vaccination
Time Frame: The 28th day after the second dose vaccination

within 14 days after each vaccination
on day 28 after boost vaccination
on day 28 after boost vaccination

Time Frame: within 14 days post each vaccination
Time Frame: Day 28 post the second vaccination
Time Frame: Day 28 post the second vaccination

within 28 d after boosting
at 14 d after boosting

Time Frame: Within 28 days after the booster dose
Time Frame: On day 14 after the booster dose

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Index	Registration No.	Journal/Registry Platform
301	ChiCTR2000029386	Chinese Medical Journal
	ChiCTR2000029386	ChiCTR
302	ChiCTR2000029741	Trials
	ChiCTR2000029741	ChiCTR
303	ChiCTR2000029777	Trials
	ChiCTR2000029777	ChiCTR
304	ChiCTR2000029972	Trials
	ChiCTR2000029972	ChiCTR
305	ChiCTR2000029993	TMR Modern Herbal Medicine
	ChiCTR2000029993	ChiCTR
306	ChiCTR2000029994	Trials

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	ChiCTR2000029994	ChiCTR
307	ChiCTR2000030043	Trials
	ChiCTR2000030043	ChiCTR
308	ChiCTR2000030703	Trials
	ChiCTR2000030703	ChiCTR
309	ChiCTR2000030759	Trials
	ChiCTR2000030759	ChiCTR
310	NCT04336254 (ChiCTR2000031319)	Trials
	NCT04336254	ClinicalTrials.gov
311	ChiCTR2000033284	Infectious Diseases of Poverty
	ChiCTR2000033284	ChiCTR
312	ChiCTR2000037524	Infectious Diseases of Poverty

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	ChiCTR2000037524	ChiCTR
313	NCT04257656	Trials
	NCT04257656	ClinicalTrials.gov
314	NCT04264533	BMJ Open
	NCT04264533	ClinicalTrials.gov
315	NCT04333589	Trials
	NCT04333589	ClinicalTrials.gov
316	NCT04334265	Trials
	NCT04334265	ClinicalTrials.gov
317	ChiCTR2000029869	Trials
	ChiCTR2000029869	ChiCTR
318	ChiCTR2000032367	Trials

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	ChiCTR2000032367	ChiCTR
319	ChiCTR2000032573	Pulmonary Circulation
	ChiCTR2000032573	ChiCTR
320	ChiCTR2000032737	Current Medical Science
	ChiCTR2000032737	ChiCTR

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DOI	Publication Title/Registration (Official/Scientific) Title
10.1097/CM9.0000000000000791	Effectiveness of glucocorticoid therapy in patients with severe coronavirus disease 2019: protocol of a randomized controlled trial A study on treatment strategies of novel coronavirus pnueumonia patients
10.1186/s13063-020-04478-w	Efficacy of chloroquine versus lopinavir/ritonavir in mild/general COVID-19 infection: a prospective, open-label, multicenter, randomized controlled clinical Efficacy of Chloroquine and Lopinavir/ Ritonavir in mild/general novel coronavirus (CoVID-19) infections: a prospective, open-label, multicenter randomized controlled
10.1186/s13063-020-04498-6	Shenhuang granule in the treatment of severe coronavirus disease 2019 (COVID-19): study protocol for an openlabel randomized controlled clinical trial A multicenter, randomized, controlled trial for integrated chinese and western medicine in the treatment of novel coronavirus pneumonia (COVID-19) based
10.1186/s13063-020-04534-5	The curative effects of shortwave diathermy on treating Novel coronavirus (COVID-19) pneumonia: A structured summary of a study protocol for a A randomized controlled trial for the Efficacy of Ultra Short Wave Electrotherapy in the treatment of Novel Coronavirus Pneumonia (COVID-19)
10.12032/TMRmhm202003084	Efficacy and safety of the combination of Liushen capsules and Arbidol in the treatment of COVID-19: protocol for a randomized, multi-center pilot study A multicenter, randomized, open-label, parallel controlled trial for the safety and efficacy of Integrated Chinese and Western Medicine in treatment of non-
10.1186/s13063-020-04693-5	Acupressure therapy and Liu Zi Jue Qigong for pulmonary function and quality of life in patients with severe novel coronavirus pneumonia (COVID-19): a

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	Liu-Zi-Jue Qigong and Acupressure Therapy for Pulmonary Function and Quality of Life in Patient with Severe novel coronavirus pneumonia (COVID-19): A
10.1186/s13063-020-04677-5	Impact of Shenfu injection on a composite of organ dysfunction development in critically ill patients with coronavirus disease 2019 (COVID-19): A structured Shen-Fu injection in the treatment of severe novel coronavirus pneumonia (COVID-19): a multicenter, randomized, open-label, controlled trial
10.1186/s13063-020-04925-8	Safety and Efficacy of Ixekizumab and Antiviral Treatment for Patients with COVID-19: A structured summary of a study protocol for a Pilot Randomized A randomized, blinded, controlled, multicenter clinical trial to evaluate the efficacy and safety of Ixekizumab combined with conventional antiviral drugs
10.1186/s13063-020-04970-3	Efficacy of Traditional Chinese Medicine, Maxingshigan-Weijing in the management of COVID-19 patients with severe acute respiratory syndrome: A structured Study for the therapeutic effect and mechanism of traditional Chinese medicine in the treatment of novel coronavirus pneumonia (COVID-19)
10.1186/s13063-020-04380-5	Safety and efficacy assessment of allogeneic human dental pulp stem cells to treat patients with severe COVID-19: structured summary of a study protocol for Safety and Efficacy Study of Allogeneic Human Dental Pulp Mesenchymal Stem Cells to Treat Severe Pneumonia of COVID-19:a Single-center, Prospective,
10.1186/s40249-021-00813-8	Efcacy of the combination of modern medicine and traditional Chinese medicine in pulmonary fbrosis arising as a sequelae in convalescent COVID-19 patients: a Study for the improvement of the quality of life of severe patients of novel coronavirus pneumonia (COVID-19) by the integrated Chinese medicine and Western medicine
10.1186/s40249-021-00836-1	Mindfulness-based online intervention on mental health and quality of life among COVID-19 patients in China: an intervention design

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	Longitudinal research project of Wuhan novel coronavirus pneumonia (COVID-19) patients and important rehabilitation factors
10.1186/s13063-020-04352-9	Evaluation of the efficacy and safety of intravenous remdesivir in adult patients with severe COVID-19: study protocol for a phase 3 randomized, double-blind, A Phase 3 Randomized, Double-blind, Placebo-controlled, Multicenter Study to Evaluate the Efficacy and Safety of Remdesivir in Hospitalized Adult Patients
10.1136/bmjopen-2020-039519	Intravenous high-dose vitamin C for the treatment of severe COVID-19: study protocol for a multicentre randomised controlled trial Vitamin C Infusion for the Treatment of Severe 2019-nCoV Infected Pneumonia: a Prospective Randomized Clinical Trial
10.1186/s13063-020-04430-y	The Mechanism and Clinical Outcome of patients with Corona Virus Disease 2019 Whose Nucleic Acid Test has changed from negative to positive, and the The Mechanism, Clinical Outcome and Therapeutic Intervention of Corona Virus Disease 2019 Patients Whose Nucleic Acids Changed From Negative to Positive
10.1186/s13063-020-04399-8	Efficacy and safety of Anluohuaxian in the treatment of patients with severe Coronavirus disease 2019- a multicenter, open label, randomized controlled study: a Efficacy and Safety of Anluohuaxian in the Treatment of Rehabilitation Patients With Corona Virus Disease 2019-A Multicenter, Open, Randomized Controlled Study
10.1186/s13063-021-05418-y	Baidu Jieduan granules, traditional Chinese medicine, in the treatment of moderate coronavirus disease-2019 (COVID-19): study protocol for an open- A multicenter, randomized, controlled trial for integrated Chinese and western medicine in the treatment of ordinary novel coronavirus pneumonia (COVID-19) based
10.1186/s13063-020-04383-2	Efficacy of Liu-zi-jue in Patients with 2019 Novel Coronavirus Pneumonia (COVID-19): structured summary of a study protocol for a randomized controlled trial

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	Rqesearch on the Application of Traditional Chinese Medicine Traditional Techniques to Prevent and Control New Coronary Pneumonia
10.1177/20458940211032125	Bufei huoxue capsules in the management of convalescent COVID-19 infection: study protocol for a multicenter, double-blind, and randomized controlled trial
	A randomized, double-blind, controlled trial for Bufei Huoxue Capsule in the treatment of convalescent COVID-19 patients
10.1007/s11596-021-2475-2	Efficacy and Safety of Washed Microbiota Transplantation to Treat Patients with Mild-to-Severe COVID-19 and Suspected of Having Gut Microbiota Dysbiosis: Study
	Clinical trial for the washed microbiota transplantation in the treatment of novel coronavirus pneumonia (COVID-19) patients suspected with gut microbiota

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Deviation in Estimated Enrollment (1 if Yes, 0 if not specified in one or more of data sources)	Absoulute Deviation by Precentage	Estimated Enrollment	Deviation in Center (1 if Yes, 0 if not specified in one or more of data sources)
		48	
		48	
		112	1
		112	
		160	1
		160	
1	925%	410	
		40	
		40	0
		40	
1	36%	128	

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		200	
		300	1
		300	
		40	
		40	
1	100%	140	
		70	
		20	
		20	
1	242.67%	514	0
		150	
		1600	1

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		1600	
1	0.22%	453	0
		452	
1	120%	308	1
		140	
		210	1
		210	
		750	
		750	
		300	1
		300	
		186	0

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		186	
		120	0
		120	
0		#BLANK	1
		60	

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Center	Deviation in Inclusion Criteria (1 if Yes, 0 if not specified in one or more of data sources)
Chongqing Public Health Medical Center	
Chongqing Public Health Medical Center	
Fifth Affiliated Hospital of Sun Yat-sen University the Ninth People's Hospital of Dongguan Zhongshan Second People's Hospital The Fifth Affiliated Hospital Sun Yat-Sen University	1
Huangshi Hospital of Traditional Chinese Medicine Tongji Hospital Tongji Medical College Huazhong University of Science and Huangshi Hospital of traditional Chinese Medicine Tongji Hospital of Huazhong University of science and technology	1
Tongji University Hospital (Huazhong University of Science and Technology, Wuhan, China) Tongji Hospital, Tongji Medical College, Huazhong University of Science and Technology	
Not specified The First Affiliated Hospital of Guangzhou Medical University Xianning Central Hospital First People's Hospital of Yunnan Province	
Huangshi Hospital of Traditional Chinese Medicine	1

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Huangshi city hospital of TCM	
The First Affiliated Hospital of Bengbu Medical College Henan Provincial People's Hospital The First Affiliated Hospital of Nanchang The first Affiliated Hospital of AnHui Medical University Anhui Provincial Hospital for Infection The First Affiliated Hospital of Bengbu	1
Xiangya Hospital of Central South University the Third Xiangya Hospital of Central South University Xiangya Hospital (West District of Wuhan Union Medical College Hospital) Third Xiangya Hospital (Sino-French New Hospital District of Wuhan Tongji Hospital)	
the First Affiliated Hospital of Wenzhou Medical University the Second Affiliated Hospital of Wenzhou Medical University The First Affiliated Hospital of Wenzhou Medical University Wenzhou city center hospital medical group The second hospital affiliated to wenzhou	1
Renmin Hospital of Wuhan University	
Renmin Hospital of Wuhan University (East Campus)	
Not specified	1
Guangzhou Women and Children's Medical Center Guangzhou Eighth People's Hospital	
Hubei Provincial Hospital of Integrated Traditional Chinese and Western Medicine	1

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Wuhan Jinyintan Hospital

Jin Yin-tan Hospital Tongji Hospital, Tongji Medical College of Huazhong University of Science & Technology	1
Not specified	
Zhongnan Hospital of Wuhan University Leishenshan Hospital of Wuhan City	1
Zhongnan Hospital of Wuhan University	
Wuhan Pulmonary Hospital, Hubei Jinyintan Hospital of Wuhan, Hubei Ezhou Central Hospital, Hubei The Second People's Hospital of Fuyang, The Second People's Hospital of Fuyang Ezhou Hospital of Traditional Chinese Medicine Ezhou Central Hospital	1
The Second People's Hospital of Fuyang Ezhou Central Hospital Huoshenshan Hospital of Wuhan Jinyintan Hospital of Wuhan	
The Second People's Hospital of Fuyang Ezhou Central Hospital Huoshenshan Hospital of Wuhan Jinyintan Hospital of Wuhan	
the Huangshi Hospital of Traditional Chinese Medicine Tongji Hospital, Tongji Medical College, Huazhong University of Science and Huangshi Hospital of Traditional Chinese Medicine Tongji Hospital of Huazhong University of science and technology	1
Not specified	

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Yueyang Integrated Traditional Chinese and Western Medicine Hospital Affiliated to Shanghai University of Traditional Chinese Medicine

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The First Affiliated Hospital of Guangzhou Medical University
Xiangzhou District People's Hospital
Third Hospital of Baotou City

Guangzhou Eighth People's Hospital 1

The First Affiliated Hospital of Guangdong Pharmaceutical University
Guangzhou Eighth People's Hospital

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Inclusion Criteria
(1) Aged 18 years or over; (2) Severe COVID-19; (3) Be willing to give informed consent.
1. Male or female over 18 years of age; 2. Novel coronavirus infection is confirmed by pathogenic detection; 3. The diagnosis of severe coronavirus pneumonia will have to meet at least one of the following criteria:
1. Age ≥ 18 years 2. Patients diagnosed with COVID-19 according to the criteria of the seventh editio n of the Treatment Plan of China for new coronavirus pneumonia (URL: http://202. 116.81.74/cache/4/03/www.nhc.gov.cn/5d9aa1423a8a577e1cc197a0d3c434d8/ce
1. Aged >= 18 years; 2. Meet all the following criteria (refer to confirmed cases in the 5th edition of the diagnosis and treatment plan): (1) Epidemiological history;
1. Hospitalized patients with 2019 novel coronavirus confirmed by SARS-CoV-2 nucleic acid testing of the respiratory specimens which showed positive results by means of real-time reverse transcription polymerase chain reaction (RT-PCR) assay.
1. Hospitalized patients with novel coronavirus pneumonia confirmed by pathogenic detection 2. Meet any one of the criteria for severe type: 1) Respiratory distress: RP >= 30/min;
(1) Age 18-65 years (2) COVID-19 nucleic acid test is positive (3) Lung CT showed multiple patchy ground glass shadows or other typical manifestations of both lungs
(1) Aged 18-65 years; (2) Virus nucleic acid test is positive; (3) Lung CT showed multiple patchy, lumpy ground glass opacity(GGO), consolidation or other typical manifestations in both lungs .
(1) Aged 18 years or older, male or female; (2)Fever, axillary temperature > 37.3℃; (3)Mild, common and severe patients with confirmed novel coronavirus infection according to the guideline (it will be updated accordingly if the guideline of NCP
1. Aged>= 18, male or female; 2. Fever, axillary temperature > 37.3 degrees; 3. Consistent with the clinical diagnosis of COVID-19 mild, normal, severe case; 4. Patients who sign informed consent form freely and voluntarily.
(1) meet the critical diagnosis criteria for severe COVID-19, (2) are aged between 20 and 80 years and are male or female, (3) have a stable condition and are conscious and cooperative in the examination, (4) volunteer to join the trial and sign the informed consent form, and

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(1) Meet NCP critical diagnosis criteria;

(2) Volunteer to participate in this study, cooperate with examination and treatment;

(3) The selected patients were basically stable, conscious and cooperative;

The patients, who are aged from 18 to 75 years old, with a confirmed or suspected diagnosis of severe or critical COVID-19, will be consecutively recruited in the study, according to the guideline on diagnosis and treatment of COVID-19 (the 7th version) issued by National Health Commission of the People's Republic of China.

1. Confirmed and suspected cases of pneumonia from novel coronavirus infection;

2. Meet the diagnostic criteria for community - acquired pneumonia;

3. Clinical classification is "heavy" or "critical";

4. Aged 18 to 75 years;

the age is 18-75 years old, regardless of sex;

COVID-19 will be diagnosed by RT-PCR or gene sequencing;

the clinical types are common type (including severe high-risk factors) (multiple lesions of both lungs, or lung imaging showing that the lesion progressed more

1. Aged 18 to 75 years old;

2. The diagnosis of new-type coronavirus pneumonia was confirmed by RT-PCR or gene sequencing according to the "Diagnosis and Treatment Scheme for Pneumonia of New Coronavirus Infection (Trial Version 6 and Updated)", and the

1. Participants are 18-85 years of age, either male or female.

2. Diagnosed as positive for severe acute respiratory syndrome coronavirus 2 (SARS-CoV-2)

3. Symptomatic. Mild (mild clinical symptoms without signs of pneumonia in chest

(1) Patients with Novel coronavirus pneumonia diagnosed according to <diagnosis and treatment scheme for pneumonia of new coronavirus infection (trial version 5)>;

(2) Inpatients aged >18 years old;

1) Adults aged 18-65 years;

2) Voluntarily participate in this clinical trial and sign the "informed consent form" or have consent from a legal representative.

3) Diagnosed with severe pneumonia of COVID-19: nucleic acid test SARS-CoV-2

Aged 18-65 years;

Voluntarily participate in this clinical trial and sign off "informed consent form";

Diagnosed with severe pneumonia of COVID: respiratory distress, RR >30 times / min; resting oxygen saturation of 93% or less; arterial partial pressure of oxygen /

(1) Diagnostic criteria for severe or critical COVID-19 Patients exhibiting positive et iological evidence of the SARS-CoV-2 nucleic acid determined using real-time poly merase chain reaction (PCR) or gene sequencing will be included. In addition, pati ents should meet at least one of the following diagnostic criteria of severe COVID-

1. The novel coronavirus pneumonia, once diagnosed as a severe or critical illness, has been cured and discharged without any pathogen or rehabilitation;

2. It conforms to the above diagnostic standards of Western medicine and TCM syndrome differentiation;

1) Over 18 and under 65 years old

2) Have a history of COVID-19 infection

3) Proficiency in Chinese

4) Be able to independently cooperate with doctors to complete various scale

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1. Subjects over 18 years old;
2. Patients with novel coronavirus pneumonia history;
3. Those who can read Chinese characters;
4. Those who can independently cooperate with doctors to complete various

1. Age ≥ 18 years at time of signing of informed consent form
2. Laboratory (reverse transcription polymerase chain reaction [RT-PCR])-confirmed COVID-19
3. Pneumonia confirmed with chest imaging

Age ≥18 years at time of signing Informed Consent Form

Laboratory (RT-PCR) confirmed COVID-19.

Lung involvement confirmed with chest imaging

Hospitalized with a SaO2/SPO2≤94% on room air or Pa02/Fi02 ratio <300mgHg

1. Adults (age 18 years or older).
2. Diagnosed with COVID-19 according to the Diagnosis and Clinical Management of 2019 novel coronavirus infected pneumonia (trial version 5, revised version).
3. Having a respiratory failure index (RIF) (arterial oxygen tension (or

≥ 18 years old;

Diagnosed as serious or critical SARI (according to the 4th version of Diagnosis and Clinical management of 2019-nCoV infected pneumonia);

Being treated in the ICU.

1. Adults 18 to 80 years, male or female.
2. After the first diagnosis and treatment of COVID-19, the nucleic acid test of respiratory specimens such as sputum or nasopharyngeal swabs, has been negative for two consecutive times (sampling time interval of at least 24 hours), in

COVID-19 has been diagnosed, and the nucleic acid test of respiratory specimens such as sputum or nasopharyngeal swabs has been negative for two consecutive times after treatment (sampling time interval of at least 24 hours);

The nucleic acid test of specimens such as sputum, throat swabs, blood, feces and

1. Confirmed COVID-19, and the nucleic acid test of respiratory specimens such as sputum or nasopharyngeal swabs is negative twice after the treatment (sampling interval is at least 24 hours);
2. Negative nucleic acid test of respiratory specimens such as sputum or

Confirmed COVID-19, and the nucleic acid test of respiratory specimens such as sputum or nasopharyngeal swabs is negative twice after the treatment (sampling interval is at least 24 hours);

Negative nucleic acid test of respiratory specimens such as sputum or

1. Patients 18–85 years of age
2. Patients with COVID-19 confirmed by SARS-CoV2 nucleic acid testing of respiratory specimens showing positive results
3. Patients providing written informed consent

1. patients with novel coronavirus pneumonia confirmed by pathogenic detection;
2. Fever, respiratory tract and other symptoms, the change of pneumonia can be seen in imaging;
3. Aged 18 to 80 years;

(1) There is no restrictions on gender and age;

(2) Patients diagnosed with mild pneumonia (slight clinical symptoms, no pneumonia manifestations on imaging), general pneumonia patients (with fever, respiratory tract symptoms, etc., imaging showed pneumonia but no multiple organ

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(1) There is no restrictions on gender and age;
(2) Patients diagnosed with mild pneumonia (slight clinical symptoms, no pneumonia manifestations on imaging), general pneumonia patients (with fever, respiratory tract symptoms, etc., imaging showed pneumonia but no multiple organ

1. Age 18 years old, male or female.
2. Patients who were diagnosed as COVID-19 according to the “The Diagnosis and Treatment Scheme of COVID-19 (The 7th Trial Edition)”.
3. The condition meets the discharge standards stipulated in “The Diagnosis and

1. Aged >= 18 years, male or female.
2. Patients who were diagnosed as COVID-19 according to the The Diagnosis and Treatment Scheme of COVID-19(The 7th Trial Edition).
3. The condition meets the discharge standards stipulated in the The Diagnosis

Patients diagnosed with mild-to-severe COVID-19 and suspected of having GMD will be recruited for this clinical trial.

1. COVID-19 patients accompanied with gastrointestinal symptoms;
2. COVID-19 patients accompanied with antibiotic-associated diarrhea

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Deviation in Exclusion Criteria (1 if Yes, 0 if not specified in one or more of data sources)
1
1
1
1

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1

For peer review only

1

1

For peer review only

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Exclusion Criteria
(1) Are allergic or intolerant to any therapeutic drugs used in this study; (2) Pregnant or lactating women; (3) Presence of severe systemic illness that may affect the effectiveness or safety evaluation for this study.
1. Patients are allergic or intolerant to therapeutic drugs; 2. Pregnancy or lactating women; 3. Researchers believe that patients may have other factors affecting the efficacy or safety evaluation of this study.
1. Patients with a history of allergy to chloroquine phosphate, lopinavir, or ritonavir 2. Patients with hematological diseases 3. Patients with end-stage liver or kidney diseases 4. Patients with arrhythmia and/or chronic heart disease
1. Patients with a history of allergy to chloroquine phosphate, lopinavir, and ritonavir; 2. Patients with hematological diseases; 3. Patients with chronic liver and kidney disease and reaching the end stage; 4. Patients with arrhythmia and chronic heart disease;
1. Pregnant and lactating women 2. Allergic to one or more substance in “Shenhuang granule” 3. Severe primary disease including malignant tumor, blood diseases, HIV, and severe liver and kidney dysfunctions
1. Pregancy or lactating women; 2. People with allergies or allergies to components of Truncated Torsion formula; 3. Patients with serious basic diseases such as malignant tumor, cirrhosis, chronic renal failure (uremic stage), hematological disease, HIV, etc.;
(1) Patients who need ICU management, (2) Positive tests for other pathogens such as Tuberculosis, Mycoplasma, (3) Patients with respiratory failure or requiring mechanical ventilation, (4) Patients with metal implants or pacemakers,
(1) Positive for other pathogens such as A, B, Mycoplasma, Adenovirus; (2) patients require mechanical ventilation; (3) People with metal or pacemaker; (4) active tuberculosis;
(1) Critical patients, who fulfill one of the following conditions: I. Respiratory failure and need mechanical ventilation; II. Shock; III. Organ failure which is other than the respiratory system that requires monitor and treatment in an intensive care unit. (2) Patients with respiratory tract infection caused by other virus except novel coronavirus;
1. Critical case with the following situation: i. Patients with respiratory failure and needing mechanical ventilation; ii. Patients with shock; iii. Patients with other organ failures and needing ICU monitoring and treatment; 2. Patients who have other respiratory tract infection;
(1) patients with complications of other serious underlying diseases, such as chronic obstructive pulmonary disease, obstructive pulmonary disease, coronary heart disease, and hypertension; (2) patients with serious mental illness; (3) patients with cognitive dysfunction who are unable to understand the trial process and

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1. Patients with other serious diseases such as lung diseases, cardiovascular and cerebrovascular diseases, hematopoietic diseases, autoimmune diseases, digestive system, or mental illness;
2. Pregnant or lactating women;
3. Participate in other forms of exercise during the trial

pregnant and breastfeeding women,
atopy or allergies to Shenfu Injection (SFI),
severe underlying disease (malignant tumor with multiple metastases, uncontrolled hemopathy, cachexia, severe malnutrition, HIV),

1. Pregnancy or lactating women;
2. People with allergies or allergies to Shenfu injection and its components;
3. Severe basic diseases that affect survival, including: uncontrolled malignant tumors with multiple metastases that cannot be removed, hematological diseases, cachexia, active bleeding, severe

severe new pneumonia infection with one of the following conditions: respiratory failure requiring mechanical ventilation, shock, ICU monitoring and treatment with other organ failure;
allergy to ixekizumab or antiviral drugs;
ALT/AST>5 times the ULN, neutrophils<0.5 × 109/L, or platelet less than 50 × 109/L;

1. Critical new pneumonia infection that meets one of the following conditions: respiratory failure occurs and requires mechanical ventilation; shock; combined organ failure requires ICU monitoring and treatment
2. Known allergies to Ixekizumab or antiviral drugs;

1. < 18 or > 85 years old.
2. Pregnancy and lactation.
3. Serious heart, liver, kidney and hematopoietic system diseases, abnormal liver or kidney function.

(1) Death is expected within 48 hours;
(2) Basic diseases such as primary immunodeficiency disease, acquired immunodeficiency syndrome, congenital respiratory malformation , congenital heart disease, gastroesophageal reflux disease, congenital pulmonary airway malformation and more with the exact evidence of bacterial

1. Patients have received other experimental treatment for COVID-19 within the last 30 days;
2. Patients have severe liver condition (e.g., Child Pugh score >=C or AST> 5 times of the upper limit);
3. Patients with severe renal insufficiency (estimated glomerular filtration rate <=30mL / min/1.73

Receive any clinical trial drug treatment for COVID-2019 within 30 days before the screening assessment;
Severe liver disease (e.g., Child Pugh score >=C or AST> 5 times of the upper limit);
Patients with known severe renal insufficiency (estimated glomerular filtration rate <=30mL /

First, diagnosis of any lung diseases, such as bronchiectasis, tuberculosis, bronchial asthma, chronic obstructive pulmonary disease (COPD), dyspnea, respiratory failure, lung cancer, or those with a history of these diseases.
Second, patients with a status of acute exacerbation of pulmonary fibrosis.

1. Patients with bronchiectasis, tuberculosis, asthma, chronic obstructive pulmonary disease, lung cancer and other primary lung diseases;
2. Progressive exacerbation of dyspnea symptoms in the past 6 months;
3. In the acute exacerbation stage of pulmonary fibrosis;

1) Have serious cognitive impairment.
2) Have serious heart, brain, lung, kidney, liver, and other medical diseases or tumors.
3) Difficult to cooperate with the questionnaire survey and intervention.

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1. Patients with severe mental illness or suicidal tendency;
2. Patients with serious heart, brain, lung, kidney, liver and other medical diseases or tumors;
3. Patients who can not cooperate with the investigation and intervention;
4. Pregnant or lactating women.

1. Physician makes a decision that trial involvement is not in patient's best interest, or patient has any condition that does not allow the protocol to be followed safely
2. Known severe liver disease (e.g. cirrhosis, ALT > 5 × upper limit of normal (ULN), or AST > 5 × ULN)

Physician makes a decision that trial involvement is not in patients' best interest, or any condition that does not allow the protocol to be followed safely.

Severe liver disease (e.g. Child Pugh score ≥ C, AST>5 times upper limit)

Pregnant or breastfeeding, or positive pregnancy test in a predose examination

1. Patients with a history of allergy to VC.
2. Patients with dyspnoea caused by cardiogenic pulmonary oedema.
3. Pregnant or lactating women.
4. Patients with an expected survival duration of less than 24 hours.

Allergic to vitamin C;

Dyspnea due to cardiogenic pulmonary edema;

Pregnant or breastfeeding;

Expected life is less than 24 hours;

1. Allergic to Favipiravjr;
2. Pregnant or lactating women
3. Uncontrolled diseases of the blood and cardiovascular system, liver or kidney.
4. History of mental disorders, drug abuse or dependence;

Those allergic to fapilavir;

Pregnant or lactating women;

Unstable liver, kidney, and heart diseases;

History of mental disorders, substance abuse or dependence;

1. Combined with severe heart, lung (diagnosed with interstitial lung disease, bronchial asthma, chronic obstructive pulmonary disease, etc.), liver and kidney disease or with endocrine, rheumatic, neurologic, malignant and other systemic diseases;
2. Have been diagnosed with connective tissue disease;

Combined with severe heart, lung (diagnosed with interstitial lung disease, bronchial asthma, chronic obstructive pulmonary disease, etc.), liver and kidney disease or with endocrine, rheumatic, neurologic, malignant and other systemic diseases;

Have been diagnosed with connective tissue disease;

1. Pregnancy and lactation
2. Severe primary diseases influencing survival, including malignant tumours, haemorrhagic disease and HIV
3. Severe liver and kidney dysfunction

1. Pregancy or lactating women;
2. People with allergies or allergies to traditional Chinese medicine
3. Severe Primary disease such as uncontrolled and metastasis malignancy, hematopathy, HIV;
4. Obstructive pneumonia caused by lung tumors, severe pulmonary interstitial fibrosis, alveolar

(1) patients with severe diseases such as cardiovascular, cerebrovascular, hematopoietic, digestive system or mental illness;

(2)pregnant and lactating women;

(3) respiratory frequency> 30 times /min, showing respiratory failure;

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(1) patients with severe diseases such as cardiovascular, cerebrovascular, hematopoietic, digestive system or mental illness;
(2) pregnant and lactating women;
(3) respiratory frequency> 30 times /min, showing respiratory failure;

1. Known or suspected allergy to the components of BFHX.
2. Acute infections, along with other viruses or bacteria.
3. Abnormal liver and kidney function tests (ALT, AST, SCr 1.5 times the upper limit of normal).
4. Participating in other drug clinical trials.

1. Known or suspected allergy to the components of BFHX.
2. Acute infections, along with other viruses or bacteria.
3. Abnormal liver and kidney function tests (ALT, AST, SCr >= 1.5 times the upper limit of normal).
4. Participating in other drug clinical trials.

critical type of COVID-19 according to the COVID-19 diagnosis and treatment guidelines in China (7th edition),
gastrointestinal perforation,
gastrointestinal obstruction.

(1) Critical type of COVID-19 according to COVID-19 diagnosis and treatment guidelines in China (7th edition);
(2) Gastrointestinal perforation;
(3) Gastrointestinal obstruction;

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Deviation in Masking (1 if Yes, 0 if not specified in one or more of data sources)	Masking	Deviation in Primary outcome (1 if Yes, 0 if not specified in one or more of data
0	Open	
	Not stated	
	Open	1
	Open	
0	Open	1
	Not stated	
0	Open	1
	Not stated	
	Open	
	Open	
	Open	1

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	Open	
0	Single	1
	Not stated	
	Open	
	Open	
0	Open	1
	#BLANK	
	Double	
	Double	
0	Double	1
	Not stated	
0	Double	1

For peer review only

	#BLANK	
	Double	
	Double	
	Single	
	Single	
	Open	
	Open	
	Open	
	Open	
	Open	
0	Open	1
	Not stated	
	Open	

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	Open	
	Double	1
	Double	
0	Single	1
	Not stated	

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Primary Outcome
the change in sequential organ failure assessment (SOFA) at 3 days after randomization SOFA score
the clinical recovery time of no more than 28 days after the completion of therapy and follow-up Length of stay Length of severe oxygenation index during treatment all-cause mortality in 28 days
clinical improvement up to 14 days after treatment 14 day outcome of the subjects, including: recovery, improvement, turning critical, death. lung CT
time to positive-to-negative conversion of COVID-19 nucleic acid test by pharyngeal swab, in days assessed at 7th, 14th ,21st and 28th days the rate of Coronary virus nucleic acid negative at 7 days, 14 days, 21 days, and 28 days after Ultra Short Wave Electrotherapy systemic inflammatory response syndrome (SIRS) scores at 7 days, 14 days, 21 days, and 28 days after Ultra Short Wave Electrotherapy
Time to major symptomatic remission: The patient's axillary temperature remains normal for >24 hours (without antipyretic drugs or corticosteroids), and the patient feels no dyspnea (for patients without dyspnea) or relief of dyspnea, oxygen saturation (SpO2) ≥ Main symptom relief time
Modified Medical Research Council (mMRC) Dyspnea Scale

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lung function
ADL
6min walk

The primary endpoint is a composite of newly developed or exacerbated organ dysfunction. This is defined as an increase in the sequential organ failure assessment (SOFA) score of two or more, indicating sepsis and involvement of at least one organ. The SOFA

pneumonia severity index (PSI)
Incidence of new organ dysfunction

change in pulmonary CT severity score (an imaging tool for assessing COVID-19, which scores on the basis of all abnormal areas involved) assessed on the 7th day, 14th day, or at discharge

Lung CT

the number of days until the clinical symptom of fever improves in the first 14 days of treatment following randomisation

The time for the positive nucleic acid of the novel coronavirus to turn negative
Incidence of deterioration
The defervescence time

Time To Clinical Improvement (TTCI)

TTCI

The main outcome measure is the treatment effect, which will measure the classification indicators, such as improvement, stabilization, and deterioration levels.

Vital Capacity
Total Vital Capacity
Forced Vital Capacity
The Predicted Forced Vital Capacity

PTSD, depression, anxiety and peace of mind

The psychological impact of COVID-19 will be assessed by the 22-item Impact of Event Scale-Revised (IES-R). IES-R is composed of three subscales and aims to measure the mean avoidance, intrusion,

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Psychological status
Sleep status

time to clinical improvement (TTCI) (censored at day 28)

Time to Clinical Improvement (TTCI)

ventilator-free days in the 28 days since admission to the ICU

Ventilation-free days

Viral nucleic acid test negative [Time Frame: 5 months]: Subjects who tested negative for nucleic acid from sputum or nasopharyngeal swabs for two consecutive times (sampling time interval of at least 24 hours).

Viral nucleic acid test negative conversion rate

1. Changes in high-resolution computer tomography of the lung
Changes in ground-glass shadows, interstitial or air nodules found on high-resolution computer tomography [Time Frame: 3 months]
2. Change in 6-minute walking distance [Time Frame: 3 months]

Changes in high-resolution computer tomography of the lung
Change in 6-minute walking distance

the proportion of negative SARS-CoV-2 nucleic acid results

14 day outcome of the subjects, including: recovery, improvement, turning critical, death.
lung CT

Length of Hospital Stay
Modify Borg Dyspnea Score

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Hospital stay
Modify Borg Dyspnea Score

CT improvement rate of lung
FAI
6MWD

St George's respiratory questionnaire score
Fatigue assessment scale
Six minute walking experiment

clinical efficacy of WMT on COVID-19 patients suspected of having GMD. (reflected by the SARS-Cov-2 infection status and the recovery of the disease, which will be defined as “improvement” (from the severe type to the moderate or mild type, or from the

SARS-Cov-2 nucleic acid test

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Secondary Outcome
proportion of mechanical ventilation use at 2 and 4 weeks, mortality at 2 and 4 weeks, and duration of hospitalization
Duration of hospitalization Proportion of mechanical ventilation use Mortality
the rate of treatment success after the completion of therapy and follow-up, the time of treatment success after no more than 28 days, the rate of serious adverse events (SAEs) during the completion of
Others (liver and kidney function, muscle enzymes, myoglobin)
mortality and rate of receiving invasive ventilator in the severe category, the improvement of oxygen saturation, and Acute Physiology and Chronic Health Evaluation (APACHE-II) score
vital signs oxygenation index Routine Blood inflammatory biomarkers
nucleic acid test rate and recovery from symptoms , vital signs assessment, treatment adverse effects, Vital capacity,
time to clinical recovey the seven-category?ordinal scores at 7 days, 14 days, 21 days, and 28 days after Ultra Short Wave Electrotherapy
(1) The time when novel coronavirus nucleic acid of the respiratory tract specimens (such as throat swabs) becomes negative for two consecutive times: Record the test results of the subject's novel coronavirus nucleic acid, and when two consecutive tests are negative
Time to COVID-19 RNA convert negative Other symptoms relief rate & time Time & Rate of inflammatory biomarkers convert normal Time to recovery
Activities of Daily Living Barthel Index (ADL-BI) Scale Patient Health Questionnaire-9 (PHQ-9) Scale Respiratory Symptoms (RS) Scale

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PHQ-9
Length of hospital stay
Respiratory symptoms
immune cells
SOFA score in total
Pneumonia severity index score
Dosage of vasoactive drugs
Ventilation free days within 28 days
28-day prognosis
SOFA
Mechanical ventilation time
Length of stay in ICU
1) Pulmonary function prognostic indicators: percentage of subjects reporting each severity level (on a 7-point scale) (https://clinicaltrials.gov/ct2/show/NCT04280705). The scale is the assessment of clinical status at the first evaluation on a specified study

Lung function
Arterial blood gas analysis

TCM Syndrome Scoring System
Coronavirus nucleic acid test
Improvements of chest X-ray (CRX)
Rate of symptom recovery
Haematological
C-reactive protein
Blood gas analysis
Urine test
1. vital signs: heart rate, blood pressure (systolic blood pressure, diastolic blood pressure). During the screening period, hospitalization every day (additional time points of D1, D4, D7 30min before injection, 2h ± 30min,
Lung lesion
Immune function
Time of SARS-CoV-2 clearance
Blood test
(1) Indices reflecting respiratory function
(2) Six-minute walking distance and Borg dyspnea score
(3) Quality of life assessment indices
(4) Evaluation standard of treatment effect in TCM syndrome

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fatigue and sleep quality
Fatigue scale-14 (FS-14) assesses the patient's level of physical fatigue and mental fatigue (Cronbach's α = 0.81) [33, 34]. The 19-item Pittsburgh Sleep Quality Index (PSQI) assesses respondents' sleep

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1. Percentage of subjects in each clinical category on the six-category ordinal scale (time frame: days 7, 14, 21, and 28)
2. Time to disease improvement, defined as the time to hospital discharge, or NEWS2 (National Early Warning Score 2) of ≤ 2
Clinical status
Time to Hospital Discharge OR NEWS2 (National Early Warning Score 2) of ≤ 2 maintained for 24 hours.
All cause mortality
1. ICU mortality, 28-day mortality.
2. Changes in SOFA scores.
3. Changes in plasma biomarkers of inflammation: including white cell counts, neutrophil counts/percent- age, lymphocyte counts/percentage,
28-days mortality
ICU length of stay
Demand for first aid measuments
Vasopressor days
Clinical cure [Time Frame: 5 months]:
1. Body temperature returned to normal for more than 3 days;
2. Lung image improved.
3. Clinical manifestation improved;

Clinical cure rate

Not Stated

Changes in compound physiological index
Changes in the scores of the St. George's Hospital Respiratory Questionnaire
Changes in modified British Medical Research Council Dyspnea Scale
the time to symptom recovery
the proportion of cases with reversion on chest CT
the time to negative SARS-CoV-2 nucleic acid results
the proportion of participants regrouped as severe cases
Pneumonia symptoms
oxygenation index
Routine Blood
major organ function
Fatigue Scale-14
Patient Health Questionnaire
vital signs
respiratory symptoms

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Fatigue scale-14
Patient Health Questionnaire
Clinical Respiratory Symptom Scale
Vital signs
(1) The evaluation results and changes of the St. George's Respiratory Questionnaire (SGRQ) score.
(2) According to the "Chinese Medicine Treatment" part of The Diagnosis and Treatment Plan (The 7th Trial Edition) and the

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1) advantages of WMT over routine treatment, as reflected by the recovery time (i.e., the time from positivity to negativity by the nasopharyngeal swab test; the clearance time of SARS-Cov-2 from feces, defined as the time from positivity to negativity by the feces
Fasting blood glucose
Liver function
Renal function
Electrolyte

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New primary outcome introduced in publiucation (1 = Yes, 0 = No)	Original primary outcome omitted in publication (1 = Yes, 0 = No)
1	1
0	1
1	0
1	1

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1	1
0	1
1	1
0	1

For peer review only



For peer review only

1	0
1	0

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Secondary outcome in registry described as primary outcome in publication (1 = Yes, 0 = No)	Primary outcome in registry described as secondary outcome in publication (1 = Yes, 0 = No)
0	0
0	0
0	1
0	1

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0	1

0	1

0	1

0	1

For peer review only



For peer review only

0	1

0	1

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Deviation in time of Primary outcome Measurement (1 if Yes, 0 if not specified in one or more of data sources)	Time of Primary Outcome Measurement
0	3 days after randomization
	Not Specified
	Time
	Time

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1	on the 7th day, 14th day, or at discharge
	Days 7 and 14
	Time Frame: 1-28 days
	Time Frame: 1-28 days

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	Time (censored at day 28) [Censored at Day 28] Time Frame: up to 28 days
	in the 28 days since admission to the ICU Time Frame: on the day 28 after enrollment
	Time Frame: 5 months Time Frame: 5 months
	Time Frame: 3 months Time Frame: 3 months Time Frame: 3 months Time Frame: 3 months
0	the baseline, before treatment on the sixth day of hospitalization and the discharge day

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Not Specified

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Index	Registration No.	Journal/Registry Platform	DOI
Protocol-313	NCT04257656	Trials	10.1186/s13063-020-04352-9
Report-128	NCT04257656	Lancet	10.1016/S0140-6736(20)31022-9
Protocol-314	NCT04264533	BMJ Open	10.1136/bmjopen-2020-039519
Report-130	NCT04264533	Ann Intensive Care.	10.1186/s13613-020-00792-3
Protocol-303	ChiCTR2000029777	Trials	10.1186/s13063-020-04498-6
Report-145	ChiCTR2000029777	Phytomedicine	10.1016/j.phymed.2021.153612
Protocol-306	ChiCTR2000029994	Trials	10.1186/s13063-020-04693-5
Report-149	ChiCTR2000029994	Integrative Medicine Research	10.1016/j.imr.2021.100796
Protocol-309	ChiCTR2000030759	Trials	10.1186/s13063-020-04970-3
Report-155	ChiCTR2000030759	Integrative Medicine Research	10.1016/j.imr.2021.100782
Protocol-315	NCT04333589	Trials	10.1186/s13063-020-04430-y

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Report-164	NCT04333589	International Immunopharmacology	10.1016/j.intimp.2021.107702
Protocol-319	ChiCTR2000032573	Pulmonary Circulation	10.1177/20458940211032125
Report-157	ChiCTR2000032573	Journal of Ethnopharmacology	10.1016/j.jep.2021.114830

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Publication Title/Registration (Official/Scientific) Title

Evaluation of the efficacy and safety of intravenous remdesivir in adult patients with severe COVID-19: study protocol for a phase 3 randomized, double-blind, placebo-controlled, multicentre trial

Remdesivir in adults with severe COVID-19: a randomised, double-blind, placebo-controlled, multicentre trial

Intravenous high-dose vitamin C for the treatment of severe COVID-19: study protocol for a multicentre randomised controlled trial

Pilot trial of high-dose vitamin C in critically ill COVID-19 patients

Shenhuang granule in the treatment of severe coronavirus disease 2019 (COVID-19): study protocol for an openlabel randomized controlled clinical trial

Traditional Chinese medicine shenhuang granule in patients with severe/critical COVID-19: A randomized controlled multicenter trial

Acupressure therapy and Liu Zi Jue Qigong for pulmonary function and quality of life in patients with severe novel coronavirus pneumonia (COVID-19): a study protocol for a randomized controlled trial

Effect of qigong exercise and acupressure rehabilitation program on pulmonary function and respiratory symptoms in patients hospitalized with severe COVID-19: a randomized controlled trial

Efficacy of Traditional Chinese Medicine, Maxingshigan-Weijing in the management of COVID-19 patients with severe acute respiratory syndrome: A structured summary of a study protocol for a randomized controlled trial

Therapeutic effects of traditional Chinese medicine (Maxingshigan-Weijing Decoction) on COVID-19: An open-label randomized controlled trial

The Mechanism and Clinical Outcome of patients with Corona Virus Disease 2019 Whose Nucleic Acid Test has changed from negative to positive, and the therapeutic efficacy of Favipiravir: A structured summary of a study protocol for a randomised controlled trial

Favipiravir in the treatment of patients with SARS-CoV-2 RNA recurrent positive after discharge: A multicenter, open-label, randomized trial

Bufei huoxue capsules in the management of convalescent COVID-19 infection: study protocol for a multicenter, double-blind, and randomized controlled trial

Efficacy and safety of Bufei Huoxue capsules in the management of convalescent patients with COVID-19 infection: A multicentre, double-blind, and randomised controlled trial

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Deviation in Estimated Enrollment (1 if Yes, 0 if not specified in one or more of data sources)	Absoulute Deviation by Precentage	Actual Enrollment (Participants Randomized)/Original Estimated Enrollment in Registry
		453
1	47.70%	237
		308
1	82%	56
		160
1	23.80%	122
		128
		128
		140
1	57.90%	59
		210

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1	73.80%	55
		120
1	9.20%	131

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Deviation in Center (1 if Yes, 0 if not specified in one or more of data sources)	Center
	Jin Yin-tan Hospital Tongji Hospital, Tongji Medical College of Huazhong University of Science & Technology Wuhan Lung Hospital
0	Not specified
	Zhongnan Hospital of Wuhan University Leishenshan Hospital of Wuhan City
	Zhongnan Hospital of Wuhan University, Leishenshan (Thunder God Mountain) Hospital, Taihe Hospital
1	Huangshi Hospital of Traditional Chinese Medicine Tongji Hospital Tongji Medical College Huazhong University of Science and Technology LaoHeKou Tradiational Chinese Medicine Hospital Leishenshan Hospital of Wuhan Tongji Hospital Wuhan Mental Health Center Huangshi Hospital of TCM (The Infectious Disease
	Huangshi Hospital of Traditional Chinese Medicine
	Huangshi Hospital of Traditional Chinese Medicine
1	the First Affiliated Hospital of Wenzhou Medical University the Second Affiliated Hospital of Wenzhou Medical University First Affiliated Hospital of Wenzhou Medical University
	Wuhan Pulmonary Hospital, Hubei Jinyintan Hospital of Wuhan, Hubei Ezhou Central Hospital, Hubei The Second People's Hospital of Fuyang, Anhui

1	Wuhan Pulmonary Hospital Ezhou Central Hospital The Third People's Hospital of Shenzhen The First Affiliated Hospital of Bengbu Medical
	Not Specified
0	Not Specified

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Deviation in Inclusion Criteria (1 if Yes, 0 if not specified in one or more of data sources)
1
1
1

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1

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Inclusion Criteria
1. Age ≥ 18 years at time of signing of informed consent form 2. Laboratory (reverse transcription polymerase chain reaction [RT-PCR])-confirmed COVID-19 3. Pneumonia confirmed with chest imaging
Eligible patients were men and non-pregnant women with COVID-19 who were aged at least 18 years and were RT-PCR positive for SARS-CoV-2, had pneumonia confirmed by chest imaging, had oxygen saturation of 94% or lower on room air or a ratio of arterial oxygen partial pressure to
1. Adults (age 18 years or older). 2. Diagnosed with COVID-19 according to the Diagnosis and Clinical Management of 2019 novel coronavirus infected pneumonia (trial version 5, revised version). (1) age ≥ 18 and < 80 years; (2) RT-PCR positive for SARS-CoV-2; (3) pneumonia confirmed by chest imaging and admission to the ICU; (3) PaO2/FiO2(P/F) < 300 mmHg.
1. Hospitalized patients with 2019 novel coronavirus confirmed by SARS-CoV-2 nucleic acid testing of the respiratory specimens which showed positive results by means of real-time reverse transcription polymerase chain reaction (RT-PCR) assay. Hospitalized patients with COVID-19 were confirmed by pathogenic detection viral diagnostic techniques (nucleic acid test). Meet any one of the criteria in the severe or critical category according to the national guideline of diagnosis and treatment
(1) meet the critical diagnosis criteria for severe COVID-19, (2) are aged between 20 and 80 years and are male or female, (3) have a stable condition and are conscious and cooperative in the examination, (1) critical diagnostic criteria for severe COVID-19 (2) aged between 20 and 80 years (3) stable condition and conscious and cooperative during the examination (4) willing to participate in the trial and sign the informed consent form
1. Participants are 18-85 years of age, either male or female. 2. Diagnosed as positive for severe acute respiratory syndrome coronavirus 2 (SARS-CoV-2) 3. Symptomatic. Mild (mild clinical symptoms without signs of pneumonia in
1. Age of 18-85 years, either male or female; 2. Diagnosed as positive for severe acute respiratory syndrome coronavirus 2 (SARS-CoV-2); 3. Symptomatic. Mild (mild clinical symptoms without signs of pneumonia
1. Adults 18 to 80 years, male or female. 2. After the first diagnosis and treatment of COVID-19, the nucleic acid test of respiratory specimens such as sputum or nasopharyngeal swabs, has been negative for two consecutive times (sampling time interval of at least

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① Over 18 years of age, both male and female;

② After the first diagnosis and treatment of COVID-19, the SARS-CoV-2 RNA test of respiratory specimens such as sputum or nasopharyngeal swabs, has been negative for two consecutive times (sampling time interval

1. Age 18 years old, male or female.

2. Patients who were diagnosed as COVID-19 according to the “The Diagnosis and Treatment Scheme of COVID-19 (The 7th Trial Edition)”.

3. The condition meets the discharge standards stipulated in “The hospitalised patients who met the following criteria:

(1)age ≥18 years;

(2) diagnosis of COVID-19 in accordance with “The Diagnosis and Treatment Scheme of COVID-19 (7th Trial Version)”;

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Deviation in Exclusion Criteria (1 if Yes, 0 if not specified in one or more of data sources)
1
1

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Exclusion Criteria	Deviation in Masking (1 if Yes, 0 if not specified in one or more of data sources)
1. Physician makes a decision that trial involvement is not in patient's best interest, or patient has any condition that does not allow the protocol to be followed safely 2. Known severe liver disease (e.g. cirrhosis, ALT > 5 × upper	
Exclusion criteria included pregnancy or breast feeding; hepatic cirrhosis; alanine aminotransferase or aspartate aminotransferase more than five times the upper limit of normal; known severe renal impairment (estimated glomerular filtration	
1. Patients with a history of allergy to VC. 2. Patients with dyspnoea caused by cardiogenic pulmonary oedema. 3. Pregnant or lactating women.	
Exclusion criteria were allergy to vitamin C, pregnancy or breastfeeding, expected survival duration < 24 h, and previous history of glucose-6-phosphate dehydrogenase deficiency or end-stage pulmonary disease. Patients who were already enrolled in	1
1. Pregnant and lactating women 2. Allergic to one or more substance in "Shenhuang granule" 3. Severe primary disease including malignant tumor, blood diseases, HIV, and severe liver and kidney dysfunctions	
1 Pregnant or lactating women 2 Allergy to herbs of Shenhuang Granule 3 History of a severe primary disease, including unresectable tumors, blood diseases (such as acute leukemia and purpura	
(1) patients with complications of other serious underlying diseases, such as chronic obstructive pulmonary disease, obstructive pulmonary disease, coronary heart disease, and hypertension;	
(1) complications due to other underlying severe diseases, such as chronic obstructive pulmonary disease, obstructive pulmonary disease, coronary heart disease, and hypertension, (2) serious mental illness,	0
1. < 18 or > 85 years old. 2. Pregnancy and lactation. 3. Serious heart, liver, kidney and hematopoietic system diseases, abnormal liver or kidney function.	
1. < 18 or > 85 years old; 2. Pregnancy and lactation; 3. Serious heart, liver, kidney and hematopoietic system diseases, abnormal liver or kidney function;	
1. Allergic to Favipiravjr; 2. Pregnant or lactating women 3. Uncontrolled diseases of the blood and cardiovascular system, liver or kidney.	

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① Allergic to favipiravir;
② Pregnant or lactating women;
③The researchers considered the patient was not suitable to participate in this clinical trial.

1. Known or suspected allergy to the components of BFHX.
2. Acute infections, along with other viruses or bacteria.
3. Abnormal liver and kidney function tests (ALT, AST, SCr 1.5 times the upper limit of normal).

(1) known or suspected allergy to the components of BFHX;
(2) acute infections caused by other viruses or bacteria;
(3) abnormal liver and kidney function tests (alanine aminotransferase [ALT], aspartate aminotransferase [AST], and

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Masking	Deviation in Primary outcome (1 if Yes, 0 if not specified in one or more of data sources)
Double	
Double	
Single	
Double	1
Open	
Open	
Open	
Not specified	
Open	
Open	1
Open	

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Open

Double

Double 1

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Primary Outcome

time to clinical improvement (TTCI) (censored at day 28)

time to clinical improvement within 28 days after randomisation.

ventilator-free days in the 28 days since admission to the ICU

invasive mechanical ventilation (IMV)-free days in 28 days (IMVFD28).

clinical improvement up to 14 days after treatment

clinical improvement

Modified Medical Research Council (mMRC) Dyspnea Scale

modified Medical Research Council (mMRC) dyspnea scale

the number of days until the clinical symptom of fever improves in the first 14 days of treatment following randomisation

the rate of symptom recovery after 14 days of treatment

Viral nucleic acid test negative [Time Frame: 5 months]: Subjects who tested negative for nucleic acid from sputum or nasopharyngeal swabs for two consecutive times (sampling

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time to achieve a consecutive twice (at intervals of more than 24 h) negative RT-PCR result for SARS-CoV-2 RNA in nasopharyngeal swab and sputum sample.

CT improvement rate of lung

FAI

6MWD

improvements in 6MWD and findings on chest computed tomography (CT) images.

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Secondary Outcome

1. Percentage of subjects in each clinical category on the six-category ordinal scale (time frame: days 7, 14, 21, and 28)
2. Time to disease improvement, defined as the time to hospital discharge, or NEWS2 (National Early Warning Score 2) of ≤ 2 maintained
the proportions of patients in each category of the six-point scale at day 7, 14, and 28 after randomisation;
all-cause mortality at day 28;
frequency of invasive mechanical ventilation;

1. ICU mortality, 28-day mortality.
2. Changes in SOFA scores.
3. Changes in plasma biomarkers of inflammation: including white cell counts, neutrophil counts/percent- age, lymphocyte counts/percentage,
28-day mortality,
organ functions and inflammatory parameters, including white blood cell counts, neutrophil counts, lymphocyte counts, procalcitonin, interleukin-6 (IL-6), and C-reactive protein (CRP).

mortality and rate of receiving invasive ventilator in the severe category,
the improvement of oxygen saturation, and
Acute Physiology and Chronic Health Evaluation (APACHE-II) score

mortality
rate of advancing to critical category
receiving an invasive ventilator in the severe category

Activities of Daily Living Barthel Index (ADL-BI) Scale
Patient Health Questionnaire-9 (PHQ-9) Scale
Respiratory Symptoms (RS) Scale
the modified Borg dyspnea scale (MBS)
fatigue Scale-14 (FS-14)
patient health questionnaire-9 scale (PHQ-9)
duration of respiratory symptoms

TCM Syndrome Scoring System
Coronavirus nucleic acid test
Improvements of chest X-ray (CRX)
Rate of symptom recovery
the time to recovery of individual symptoms,
TCM syndrome scores,
the time of conversion of SARS-CoV-2 RNA assay,
hospitalization days,

Clinical cure [Time Frame: 5 months]:
1. Body temperature returned to normal for more than 3 days;
2. Lung image improved.
3. Clinical manifestation improved;

the changes of blood routine and CRP (C-reactive protein);
the count and proportion of T lymphocyte subsets in peripheral blood and the changes of cytokines

(1) The evaluation results and changes of the St. George's Respiratory Questionnaire (SGRQ) score.
(2) According to the "Chinese Medicine Treatment" part of The Diagnosis and Treatment Plan (The 7th Trial Edition) and the "Recommendation on Chinese medicine syndrome scores,
St. George's Respiratory Questionnaire (SGRQ) score,
Borg-Dyspnea Scale scores,
Fatigue Assessment Inventory (FAI) scores

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New primary outcome introduced in report (1 = Yes, 0 = No)	Original primary outcome omitted in report (1 = Yes, 0 = No)
1	1
0	0

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0 0

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Secondary outcome in protocol described as primary outcome in report (1 = Yes, 0 = No)	Primary outcome in protocol described as secondary outcome in report (1 = Yes, 0 = No)
0	0
1	1

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0 1

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Deviation in time of Primary outcome Measurement (1 if Yes, 0 if not specified in one or more of data sources)	Time of Primary Outcome Measurement
	Time (censored at day 28)
	Time
	14 days
1	
	up to 75 days
	The assessments of the clinical scales will be performed at three points (before treatment, the 7th day during hospitalization, and
1	The outcomes were evaluated at two points (pretreatment and discharge) to reflect the changes in dyspnea and clinical

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Index	Registration No.
901-1	ChiCTR2000029606 (Case Series)
901-2	ChiCTR2000029606-Report2 (Non-randomized)
902	ChiCTR2000029978 (Retrospective)
903	ChiCTR2000030627 (Case Series)
904	ChiCTR2000033320 (Retrospective Cohort)
905	ChiCTR2000034794 (Observational study)
906	ChiCTR2000029601-Report3 (Retrospective Cohort)
911	ChiCTR2000031955 (Cross-sectional)

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Journal/Registry Platform	DOI
Front. Med.	10.1007/s11684-020-0810-9
Clinical and Translational Medicine	10.1002/ctm2.297
上海中医药杂志	10.16305/j.1007-1334.2020.05.095
Chin Crit Care Med，	10.3760/cma.j.cn121430-20200810-00568
Journal of Integrative Medicine	10.1016/j.joim.2020.10.002
Inflammation Research	10.1007/s00011-020-01429-8
Pharmacological Research	10.1016/j.phrs.2020.105127
Front. Public Health,	10.3389/fpubh.2021.722604

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Publication Title/Registration (Official/Scientific) Title

Clinical study using mesenchymal stem cells for the treatment of patients with severe COVID-19

Evaluation of the safety and efficacy of using human menstrual blood-derived mesenchymal stromal cells in treating severe and critically ill COVID-19 patients: An exploratory clinical trial

308 例新型冠状病毒肺炎病例回顾性分析及抗疫强身功运动处方的临床应用方案

恢复期血浆治疗在新型冠状病毒肺炎中的应用效果分析

Effects of Tanreqing Capsule on the negative conversion time of nucleic acid in patients with COVID-19: A retrospective cohort study

Relationship between the dynamic changes of serum 2019-nCoV IgM/IgG and patient immunity after 6 month hospital discharge

Hanshiyi Formula, a medicine for Sars-CoV2 infection in China, reduced the proportion of mild and moderate COVID-19 patients turning to severe status: A cohort study

Effect of Knowledge/Practice of COVID-19 Prevention Measures on Return-to-Work Concerns; Attitudes About the Efficacy of Traditional Chinese Medicine: Survey on Supermarket Staff in Huanggang, China

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PRISMA 2020 for Abstracts Checklist

Section and Topic	Item #	Checklist item	Reported (Yes/No)
TITLE			
Title	1	Identify the report as a systematic review.	Yes
BACKGROUND			
Objectives	2	Provide an explicit statement of the main objective(s) or question(s) the review addresses.	Yes
METHODS			
Eligibility criteria	3	Specify the inclusion and exclusion criteria for the review.	Yes
Information sources	4	Specify the information sources (e.g. databases, registers) used to identify studies and the date when each was last searched.	Yes
Risk of bias	5	Specify the methods used to assess risk of bias in the included studies.	Yes
Synthesis of results	6	Specify the methods used to present and synthesise results.	Yes
RESULTS			
Included studies	7	Give the total number of included studies and participants and summarise relevant characteristics of studies.	Yes
Synthesis of results	8	Present results for main outcomes, preferably indicating the number of included studies and participants for each. If meta-analysis was done, report the summary estimate and confidence/credible interval. If comparing groups, indicate the direction of the effect (i.e. which group is favoured).	Yes
DISCUSSION			
Limitations of evidence	9	Provide a brief summary of the limitations of the evidence included in the review (e.g. study risk of bias, inconsistency and imprecision).	No, because this is covered in “Strengths and limitations of this study” section.
Interpretation	10	Provide a general interpretation of the results and important implications.	Yes
OTHER			
Funding	11	Specify the primary source of funding for the review.	No, the review was not funded.
Registration	12	Provide the register name and registration number.	No, the review was not registered.

From: Page MJ, McKenzie JE, Bossuyt PM, Boutron I, Hoffmann TC, Mulrow CD, et al. The PRISMA 2020 statement: an updated guideline for reporting systematic reviews. BMJ 2021;372:n71. doi: 10.1136/bmj.n71

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PRISMA 2020 for Abstracts Checklist

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PRISMA 2020 Checklist

Section and Topic	Item #	Checklist item	Location where item is reported
TITLE			
Title	1	Identify the report as a systematic review.	Page 1
ABSTRACT			
Abstract	2	See the PRISMA 2020 for Abstracts checklist.	Page 2~3
INTRODUCTION			
Rationale	3	Describe the rationale for the review in the context of existing knowledge.	Page 5~6
Objectives	4	Provide an explicit statement of the objective(s) or question(s) the review addresses.	Page 5~6
METHODS			
Eligibility criteria	5	Specify the inclusion and exclusion criteria for the review and how studies were grouped for the syntheses.	Page 7; Supplementary 1
Information sources	6	Specify all databases, registers, websites, organisations, reference lists and other sources searched or consulted to identify studies. Specify the date when each source was last searched or consulted.	Page 7
Search strategy	7	Present the full search strategies for all databases, registers and websites, including any filters and limits used.	Supplementary 1
Selection process	8	Specify the methods used to decide whether a study met the inclusion criteria of the review, including how many reviewers screened each record and each report retrieved, whether they worked independently, and if applicable, details of automation tools used in the process.	Page 8; Supplementary 1
Data collection process	9	Specify the methods used to collect data from reports, including how many reviewers collected data from each report, whether they worked independently, any processes for obtaining or confirming data from study investigators, and if applicable, details of automation tools used in the process.	Page 7~8
Data items	10a	List and define all outcomes for which data were sought. Specify whether all results that were compatible with each outcome domain in each study were sought (e.g. for all measures, time points, analyses), and if not, the methods used to decide which results to collect.	Page 7~8
	10b	List and define all other variables for which data were sought (e.g. participant and intervention characteristics, funding sources). Describe any assumptions made about any missing or unclear information.	Page 7~8
Study risk of bias assessment	11	Specify the methods used to assess risk of bias in the included studies, including details of the tool(s) used, how many reviewers assessed each study and whether they worked independently, and if applicable, details of automation tools used in the process.	Page 8
Effect measures	12	Specify for each outcome the effect measure(s) (e.g. risk ratio, mean difference) used in the synthesis or presentation of results.	Not applicable
Synthesis methods	13a	Describe the processes used to decide which studies were eligible for each synthesis (e.g. tabulating the study intervention characteristics and comparing against the planned groups for each synthesis (item #5)).	Page 7~8; Supplementary 1
	13b	Describe any methods required to prepare the data for presentation or synthesis, such as handling of missing summary statistics, or data conversions.	Not applicable
	13c	Describe any methods used to tabulate or visually display results of individual studies and syntheses.	Page 7~8
	13d	Describe any methods used to synthesize results and provide a rationale for the choice(s). If meta-analysis was performed, describe the model(s), method(s) to identify the presence and extent of statistical heterogeneity, and software package(s) used.	Not applicable
	13e	Describe any methods used to explore possible causes of heterogeneity among study results (e.g. subgroup analysis, meta-regression).	Not applicable

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PRISMA 2020 Checklist

Section and Topic	Item #	Checklist item	Location where item is reported
	13f	Describe any sensitivity analyses conducted to assess robustness of the synthesized results.	Not applicable
Reporting bias assessment	14	Describe any methods used to assess risk of bias due to missing results in a synthesis (arising from reporting biases).	Not applicable
Certainty assessment	15	Describe any methods used to assess certainty (or confidence) in the body of evidence for an outcome.	Not applicable
RESULTS			
Study selection	16a	Describe the results of the search and selection process, from the number of records identified in the search to the number of studies included in the review, ideally using a flow diagram.	Figure 1
	16b	Cite studies that might appear to meet the inclusion criteria, but which were excluded, and explain why they were excluded.	Supplementary 1
Study characteristics	17	Cite each included study and present its characteristics.	Table 1
Risk of bias in studies	18	Present assessments of risk of bias for each included study.	Figure 2
Results of individual studies	19	For all outcomes, present, for each study: (a) summary statistics for each group (where appropriate) and (b) an effect estimate and its precision (e.g. confidence/credible interval), ideally using structured tables or plots.	Page 10~12
Results of syntheses	20a	For each synthesis, briefly summarise the characteristics and risk of bias among contributing studies.	Page 10~12
	20b	Present results of all statistical syntheses conducted. If meta-analysis was done, present for each the summary estimate and its precision (e.g. confidence/credible interval) and measures of statistical heterogeneity. If comparing groups, describe the direction of the effect.	Not applicable
	20c	Present results of all investigations of possible causes of heterogeneity among study results.	Not applicable
	20d	Present results of all sensitivity analyses conducted to assess the robustness of the synthesized results.	Not applicable
Reporting biases	21	Present assessments of risk of bias due to missing results (arising from reporting biases) for each synthesis assessed.	Not applicable
Certainty of evidence	22	Present assessments of certainty (or confidence) in the body of evidence for each outcome assessed.	Not applicable
DISCUSSION			
Discussion	23a	Provide a general interpretation of the results in the context of other evidence.	Page 14~15
	23b	Discuss any limitations of the evidence included in the review.	Page 15~16
	23c	Discuss any limitations of the review processes used.	Page 15~16
	23d	Discuss implications of the results for practice, policy, and future research.	Page 16~17
OTHER INFORMATION			
Registration and protocol	24a	Provide registration information for the review, including register name and registration number, or state that the review was not registered.	Page 19
	24b	Indicate where the review protocol can be accessed, or state that a protocol was not prepared.	Page 7 Supplementary 1
	24c	Describe and explain any amendments to information provided at registration or in the protocol.	Supplementary 1

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PRISMA 2020 Checklist

Section and Topic	Item #	Checklist item	Location where item is reported
Support	25	Describe sources of financial or non-financial support for the review, and the role of the funders or sponsors in the review.	Page 19
Competing interests	26	Declare any competing interests of review authors.	Page 19
Availability of data, code and other materials	27	Report which of the following are publicly available and where they can be found: template data collection forms; data extracted from included studies; data used for all analyses; analytic code; any other materials used in the review.	Dataset of the review is provided with the manuscript

From: Page MJ, McKenzie JE, Bossuyt PM, Boutron I, Hoffmann TC, Mulrow CD, et al. The PRISMA 2020 statement: an updated guideline for reporting systematic reviews. BMJ 2021;372:n71. doi: 10.1136/bmj.n71
For more information, visit: http://www.prisma-statement.org/

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